Positive Psychiatry, Psychotherapy and Psychology

Clinical Applications for Positive Mental Health

Erick Messias
Hamid Peseschkian
Editors

Second Edition

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ISBN 978-3-031-94644-8 ISBN 978-3-031-94645-5 (eBook)
https://doi.org/10.1007/978-3-031-94645-5

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The editors dedicate this book in gratitude and admiration to the three "founding fathers" of the contemporary Positive Mental Health movement.

May their concepts continue to improve the lives of people around the globe and inspire young professionals to follow in their footsteps.



Foreword: Exciting Developments in Positive Mental Health

Five years ago, I felt honored to be invited to write the Foreword titled "Positive Mental Health" for the Textbook on *Positive Psychiatry, Psychotherapy and Psychology: Clinical Applications*, co-edited by Erick Messias, Hamid Peseschkian, and Consuelo Cagande. It was an outstanding book that brought together major authors from various countries and was very well received. Since then, there has been considerable progress in this fast-growing field. Also, personally, I have had wonderful opportunities to work with Drs. Peseschkian and Messias in several ways. We jointly participated in various conferences of large organizations including the World Psychiatric Association and the American Psychiatric Association. I was particularly thrilled to be the keynote speaker in the annual congress of the World Association for Positive and Transcultural Psychotherapy organized by its President, Dr. Peseschkian, in Istanbul on March 21-23, 2024. That was one of the most inspiring conferences I have participated in, with over 300 attendees from different professional backgrounds and from numerous countries and various cultures across the globe. The science presented and discussed was of high quality and so was the focus on strong positive perspectives, notwithstanding the global tensions. It is this wonderful perspective that is represented in this new book.

I am delighted to write the Foreword for the Second edition of the textbook *Positive Psychiatry, Psychotherapy and Psychology: Clinical Applications for Positive Mental Health*, co-edited by Hamid Peseschkian and Erick Messias. It highlights the progress in the field that is continuing to grow. This new edition has 55 chapters from 75 authors residing in 19 countries, including Austria, Brazil, Bulgaria, China, Cyprus, Czech Republic, Ethiopia, Georgia, Germany, India, Kosovo, Poland, Portugal, Romania, Russia, Türkiye, Ukraine, the United Kingdom, and the United States. Twenty-one chapters are on new topics that were not covered in the first edition, so this is not only a revised but also an enlarged second edition. During the last five years, a lot has changed in the outside world, including the needs of our patients and the world of positive mental health.

The notion of positive mental health goes all the way back to Aristotle's eudaimonic well-being, which is different from the hedonic approach that refers to subjective well-being, life satisfaction, happiness, and positive/negative affect. According to Ryff, eudaimonic well-being, which leads to human flourishing, is a multi-dimensional construct that includes self-acceptance, positive relations with others, environmental mastery, autonomy, purpose in

life, and personal growth. Moving on to the beginning of the twentieth century, the President of the American Philosophical Association was a philosopher, a physician, and a psychologist - William James. He proposed the notion of a "mind-cure," conceptualized as the restorative powers of positive emotions and beliefs. Positive psychotherapy was proposed in 1977 by Nossrat Peseschkian, an Iran-born, German-trained neuropsychiatrist and psychotherapist.

He integrated humanistic approach with therapeutic alliance, psychodynamic understanding of mental illnesses, and a consideration of culture, work, and environment. This work has been continued but also reformatted to fit the rapidly changing world featuring globalization and advances in technology, by Nossrat Peseschkian's son, Hamid Peseschkian, in collaboration with Erick Messias. They are focusing on pragmatic, self-help, and goal-oriented short-term psychotherapy based on transcultural perspectives. In the late 1990s, Martin Seligman, President of the American Psychological Association, launched and popularized the modern positive psychology movement.

In 2012–2013, when I was the president of the APA, I was struck by the fact that psychiatry had paid little attention to the positive aspects of mental health. On googling the term positive psychology, I got thousands of citations, but when I did the same for positive psychiatry, there was not a single one. In some other branches of medicine, especially cardiology and oncology, there were numerous papers on resilience, optimism, social engagement, and other positive traits. Yet very few of such papers were published in psychiatric journals. Similarly, psychiatric textbooks were mostly restricted to psychiatric disorders and risk factors, rarely describing wellness and protective factors. I made positive psychiatry my APA Presidential Theme. I defined positive psychiatry as the science and practice of psychiatry that seeks to understand and promote well-being through assessments and interventions aimed at enhancing positive psychosocial factors and mental wellness. In 2015, we published the first handbook on positive psychiatry.

This new second edition of the textbook on *Positive Psychiatry, Psychotherapy and Psychology*, co-edited by Erick Messias and Hamid Peseschkian, is divided into six parts. Part I focuses on "Basic Concepts, Background, and History" and includes chapters on Introductions to Positive Psychiatry, Positive Psychotherapy, and Positive Psychology.

Part II addresses "Staying Positive through Life" and includes chapters on Positive Child and Adolescent Psychiatry, Positive Psychiatry in Midlife, Professional Well-Being, Successful Aging, Life Balance with Positive Psychotherapy, and Death and Grieving with Positive Psychotherapy.

Part III is titled "Psychiatric and Psychosomatic Disorders" and contains chapters on Positive Interventions in Depression, Positive Interventions in Anxiety Disorders, Positive Interventions in Schizophrenia and Psychotic Disorders, Positive Psychotherapy and Eating Disorders, Positive Psychotherapy in PTSD and Post-traumatic Growth, Positive Psychosomatics, Positive Child and Adolescent Psychotherapy, Positive Psychotherapy and Autism Spectrum Disorders, Treatment of Chronic Pain Patients with Positive

Psychotherapy, and Positive Communication in the Therapeutic-Medical Relationship.

Part IV includes chapters on Special Settings and Populations, Minorities and Culture: Positive Psychiatry and Psychology Perspectives, Positive Psychotherapy in Different Cultures, Positive Sports Psychiatry, Positive Family and Marital Therapy, Positive Pedagogy and Counselling, Positive Group Psychotherapy, Positive Psychotherapy in Organizational and Leadership Coaching, Psychotherapeutic Work with Men, Positive Psychotherapy in Virtual and Remote Settings (Online Therapy), Suicide from the Perspective of Positive Psychiatry and Positive Psychotherapy, Particularities of the Therapeutic Relationship in Positive Child and Adolescent Psychotherapy, Trauma Counseling and Crisis Interventions Based on Positive Psychotherapy, and Sexual Counselling and Sexual Education Using Methods of Positive Psychotherapy.

Part V is comprised of chapters on "Theoretical Foundations and Training" with titles of Theoretical Foundations and Roots of Positive Psychotherapy, The First Interview in Positive Psychotherapy, Conflict Model of Positive Psychotherapy, Using Stories, Anecdotes, and Humor in Positive Psychotherapy, Supervision in Positive Psychotherapy, Spirituality and Religion in Positive Psychology and Positive Psychiatry, Positive Psychotherapy as an Existentialism, The Development of Positive Psychotherapy in Dialogue with Other Psychotherapeutic Methods, Positive Psychotherapy and Meaning of Life, and Positive Interpretation as a Tool in Psychotherapy.

The final Part VI titled "Emerging Topics and Contemporary Issues" includes chapters on Humor in Positive Psychotherapy and Psychiatry, Climate Crisis and Mental Health, Dealing with the Covid-19 Pandemic with Positive Psychotherapy, Understanding the LGBTQ Community's Needs in Therapy from a Positive Psychotherapy Perspective Pathways to Group Trauma Intervention Using Positive Psychotherapy: A Practical Experience with Severely Traumatized War Survivors, The Challenge of Mental Health Professionals Being a Refugee—and Working with Refugees, War, Working with Soldiers, Separated Families, Positive Psychotherapy and Dealing with Sexual Violence, Rape, and Trauma, Transgenerational Trauma with Positive and Transcultural Psychotherapy, Positive Psychotherapy in Promoting Resilience and Hardiness, Facilitating Immigrant Adaptation Through Positive Psychotherapy, Positive Psychotherapy Approach for Patients with Personality Disorders, and the Challenges of Academic Supervision of Psychology Students.

At the core of positive mental health are positive or strength-based psychosocial factors like wisdom, resilience, optimism, social support, purpose or meaning in life, spirituality, self-efficacy, personal mastery, and coping ability in people with mental illnesses or physical disabilities as well as in the community at large. Positive psychiatry, psychology, and psychotherapy is an evidence-based approach to understanding normal behavior as well as psychopathology and to improving well-being by evaluating and enhancing positive psychosocial factors in people with and without psychiatric or physical illnesses.

It is not surprising that people with serious mental illnesses tend to have lower levels of positive psychosocial factors possibly due to a bidirectional relationship between psychosocial adversity and psychopathology. Yet, several studies in recent years have demonstrated that these positive factors may serve to neutralize the adverse impact of negative psychosocial factors in these patients. Subsets of patients with serious mental illnesses experience recovery or sustained remission after decades of severe psychopathology.

Today's healthcare system often emphasizes disorders and risk factors over individuals' strengths, protective factors, and overall well-being. As global healthcare needs rise, this deficit-focused approach widens the gap between available services and demand. Evidence suggests that fostering positive psychosocial attributes can reduce mental illness risk, improve long-term outcomes, and aid recovery for those with serious mental health illnesses. Importantly, these benefits extend to the general population.

Wisdom encompasses empathy, compassion, emotional regulation, self-reflection, acceptance of diverse perspectives, decisiveness, wise advising, and spirituality. The San Diego Wisdom Scale measures this trait, which is associated with improved health, well-being, and societal wellness. Wisdom benefits not only individuals but also promotes harmony in communities. Interventions targeting specific components of wisdom—i.e., empathy, emotional regulation, and spirituality—show significant effectiveness. A number of published randomized controlled trials have reported substantial gains in these wisdom components with medium to large effect sizes.

Resilience, described as the capacity to adapt and bounce back from adversity, is a valuable protective factor. Resilience is linked to better health perception, immune function, and reduced severity of serious mental illnesses such as schizophrenia, major depressive disorder, and bipolar disorder. Meta-analyses show that resilience training helps manage stress and depression. Mindfulness and mixed interventions are particularly effective, helping individuals build resilience by handling moderate stress.

Research also highlights optimism's link to health benefits. A number of studies have found that higher optimism predicts greater likelihood of aging healthily. Optimism also correlates with healthier behaviors and lower suicidal ideation, even after controlling for depressive symptoms and other relevant factors. Group-based optimism programs have shown success in boosting optimism and other psychological metrics, with positive cognitive behavioral therapy (CBT) achieving better engagement and results compared to traditional CBT.

Religiosity, related to organized practices, and spirituality, characterized by transcendent beliefs, positively impact mental health, reducing depressive symptoms and substance abuse. Studies suggest that religious and spiritual practices aid in coping with mental health challenges, with reported benefits of using religious and spiritual interventions.

Meaning in life supports mental and physical well-being. Research in older adults links life meaning with better cognitive and physical function and reduced psychomotor slowing over time. Interventions that foster purpose or meaning in life, such as mindfulness and narrative therapy, offer moderate to strong benefits. Mindfulness enhances present-moment awareness,

while narrative programs encourage individuals to reflect on their life experiences, both bolstering a sense of purpose.

Cultural Dimensions of Positive Global Mental Health: While positive mental health is universal, its perception varies across cultures. Eastern cultures emphasize harmony and interconnectedness, whereas Western societies prioritize individual achievement. African cultures, such as those embracing Ubuntu, stress community and social relationships. Tailoring mental health programs to cultural values enhances intervention success and fosters inclusivity in global mental health strategies.

Social Determinants of Health (SDoHs): In recent years, I have been focusing on social determinants of health (SDoHs), especially mental health. SDoHs are non-medical factors in life that influence our health and well-being. They are the conditions in which we are born, grow, live, work, and age. They also include the relationships we have, and even broader forces like economic policies, racism, and climate change. These factors shape everything from our daily lives to our long-term health. For example, growing up in poverty or facing discrimination can increase our chances of developing both physical and mental health issues, while living in a supportive community or having access to healthy food can enhance our well-being. The SDoHs don't just affect whether we get sick but also how well we recover and how long we live. Some SDoHs are things that we encounter daily. For instance, having a stable home, access to healthy and affordable food, reliable transportation, and even internet access are all critical to maintaining good health. On the flip side, negative experiences like childhood abuse, homelessness, or discrimination can set the stage for health struggles later in life. Unfortunately, the reality is, SDoHs have a greater impact on our health than many of the medical risk factors we often focus on, such as diabetes or hypertension. By addressing these social factors, we can improve not just individual health but also the health of entire communities. Imagine a world where everyone has access to nutritious food, clean water, safe home, and a supportive network—these are the building blocks of a healthy life. And while many SDoHs have negative impacts, there are positive ones too. For instance, strong social connections, community support, and a sense of belonging can improve mental health and even help people live longer. Other positive SDoHs include being married or having a partner; parental care for children and supporting caregivers for older adults, especially those with dementia; religiosity; ethnic or cultural density for minoritized communities; and overall compassionate, age-friendly, and dementia-friendly communities.

Adverse SDoHs like poverty and discrimination produce toxic stress which leads to increased level of inflammation in the body and some epigenetic changes in the expression of healthy genes. These adverse biological effects can have an impact on the chemistry, function, and even structure of the brain and several other organs in the body. The person then gets sick which further worsens his social relationships as well as his finances. In other words, there is a vicious circle with the social factors worsening biology which leads to worse health and more social problems. In contrast, positive SDoHs such as supportive social connections have a positive impact on biology, thereby promoting well-being.

In the last 25 years, loneliness has become a major problem in the United States as well as in many other countries. Indeed, there seems to be a loneliness pandemic associated with increased general mortality from a number of physical illnesses including heart disease, diabetes, obesity, as well as dementia, major depressive disorder, and deaths of distress from suicides and opioid abuse. Loneliness means subjective distress due to perceived social isolation. Thus, there is a clear need for increasing positive social connections. There are ways in which we can achieve that goal. In essence, paying attention to SDoHs is one of the most powerful ways to enhance the well-being of everyone. By improving the conditions in which people live, we can create healthier, more resilient, compassionate, and wiser communities. And that's something we all benefit from.

The Future: The future of psychiatry is positive psychiatry. Positive psychiatry has the potential to revolutionize the management of people suffering from psychopathology, particularly those afflicted with severe mental illnesses, and in some cases, reduce their incidence. As the world population continues to rise and the gap between healthcare needs and supply increases at a faster pace, a change in the healthcare system is unquestionably and urgently warranted. A system that focuses on the prevention of mental and physical illnesses by developing and implementing broad policies that promote positive psychosocial factors such as wisdom, resilience, and positive social connections is not merely desirable—it is essential for the human species to survive and thrive. This textbook is a step in that vitally important direction.

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Introduction to the Second Edition

When we worked on the first edition of this textbook in 2019, it was with a deep sense of optimism and hope—not only for the burgeoning positive mental health movement but for humanity as a whole. That first edition was released in early 2020, a time when the world seemed poised for progress and connection.

Yet, the years that followed challenged our optimism in unprecedented ways. The COVID-19 pandemic swept across the globe in 2020, leaving profound physical, emotional, and social scars. In 2022, a large-scale military invasion in Eastern Europe shook the international order, and in 2024, the devastating attack in the Middle East marked one of the darkest chapters in recent history. In many regions of the world, there are currently military and other conflicts that may not always make the headlines but bring daily suffering to thousands of people. Amid these tragedies, societies simultaneously grew more interconnected through virtual communication while becoming more divided along national and ideological lines. The resurgence of nationalism, anti-immigrant rhetoric, and the rapid spread of misinformation through social media underscored the complexities and fragilities of our global society.

One might argue that our initial optimism was misplaced. We, however, strongly disagree. These harrowing events serve as critical reminders of the enduring need for positive mental health. Understanding human flourishing is not a luxury confined to moments of peace; it is a necessity, particularly in times of crisis. Positive mental health does not shy away from the harsh realities of life; rather, it equips us to confront them with resilience, courage, and purpose.

The past five years have also been a time of profound advancement in the fields of positive psychiatry, positive psychotherapy, and positive psychology. To reflect these developments, nearly all chapters in this second edition have been revised and updated, and 22 new chapters have been added, including a dedicated section on emerging issues such as war, trauma, and global health crises. These challenges have also spurred innovative solutions, such as virtual psychotherapy and telehealth, which have expanded access to mental health care worldwide.

In the first edition, we collaborated with 48 authors from 13 countries. This second edition features contributions from 75 authors representing 19 countries. Among these voices are those from nations as diverse as China, Ukraine, Ethiopia, and Brazil, as well as displaced professionals now working

in countries like Portugal, Türkiye, the United Kingdom, and Poland. This rich diversity is mirrored in the editors' locations, one in Wiesbaden, Germany, and the other in St. Louis, USA. We extend our heartfelt thanks to Dr. Consuelo Cagande, who co-edited the first edition and authored two chapters in this second volume.

This collaboration spans three professional perspectives: **positive psychiatry**, **positive psychotherapy**, and **positive psychology**.

Positive Psychiatry, as introduced by Dilip Jeste, integrates tools from positive psychology and psychotherapy to foster well-being in individuals with psychiatric disorders. Recent research highlights its impact, such as the role of purpose in reducing cognitive decline among older adults and the benefits of positive reminders in mitigating PTSD and suicidality among youth.

Positive Psychotherapy, founded by Nossrat Peseschkian in the 1970s, continues to grow globally, with its world association now active in 66 countries and training programs in 22 regions of the world. As a humanistic-psychodynamic method that is culturally sensitive, its concepts and tools have proven effective across diverse cultural and clinical settings.

Positive Psychology, championed by Martin Seligman, has extended its influence beyond clinical settings into education and workplaces. During the COVID-19 pandemic, its principles helped foster resilience, courage, gratitude, and connection, even in times of profound uncertainty. Emerging technologies like AI-driven chatbots now incorporate positive psychology principles to support mental well-being.

This edition is organized into 6 parts, encompassing 55 chapters. Each part introduces key themes and highlights the contributions of our diverse authors.

We must also acknowledge the linguistic diversity of our contributors. For many authors, English is not their first language, yet they have skillfully shared their expertise, drawn from distinct cultural and professional contexts. Their dedication reflects the universal relevance of positive mental health and its potential to bridge differences. Their work exemplifies the shared goal of fostering human flourishing across borders, languages, and traditions.

We hope readers approach this work with an appreciation for its global and multidisciplinary perspectives. While perfection may be unattainable, we believe this edition offers a coherent and comprehensive resource that reflects the challenges and opportunities of our time.

This textbook stands as both a testament to our belief in the power of collaboration and an invitation to join us in advancing the understanding of positive mental health.

We are deeply grateful to our 73 colleagues and co-authors who, despite their busy schedules, contributed to this endeavor with their invaluable insights and experiences. Together, we have created a work that demonstrates the profound strength of collective knowledge and shared purpose.

St. Louis, MO, USA — Erick Messias
Wiesbaden, Germany — Hamid Peseschkian
May 2025

Overview of the Chapters

Part I: Basic Concepts, Background, and History

This section includes three chapters that introduce foundational principles and historical perspectives.

- **Chapter 1** defines Positive Psychiatry, developed by Dilip Jeste, as the application of Positive Psychology principles to mental health care. It provides a historical overview and illustrates its use in conditions such as schizophrenia, depression, child psychiatry, and PTSD.
- **Chapter 2** explores the history and theory of Positive Psychotherapy (PPT), developed by Nossrat Peseschkian. It outlines the key concepts and tools of this humanistic-psychodynamic approach, emphasizing its versatility and transcultural applicability.
- **Chapter 3** outlines Positive Psychology, emphasizing the PERMA model (Positive Emotion, Engagement, Relationships, Meaning, and Accomplishment) and learned optimism as developed by Martin Seligman.

Part II: Staying Positive Through Life

This section follows the lifespan and includes six chapters on resilience, professional well-being, and life's final stages.

- **Chapter 4** identifies 12 wellness domains in Positive Child and Adolescent Psychiatry, advocating for prevention strategies to foster resilience and psychological strength.
- **Chapter 5** examines resilience factors in midlife, including purpose, spirituality, humor, and emotional objectivity, offering insights for navigating the midlife crisis.
- **Chapter 6** discusses professional well-being, addressing burnout and applying Erikson's developmental model to career challenges.
- **Chapter 7** reviews the concept of Successful Aging, incorporating studies on centenarians and advancements in technology.
- **Chapter 8** advocates for life balance over work-life balance using the PPT Balance Model.

- **Chapter 9** applies positive psychotherapy to the topics of death, dying, and grief, with cross-cultural examples from Africa, Asia, and Europe.

Part III: Psychiatric and Psychosomatic Disorders

This section includes ten chapters addressing various mental health conditions.

- **Chapter 10** introduces Positive Psychiatry for depression, highlighting the integration of biological and psychological approaches to enhance resilience and well-being.
- **Chapter 11** explores Positive Psychology Interventions (PPIs) for anxiety disorders, focusing on psychosocial factors, sleep, and physical activity.
- **Chapter 12** discusses the use of PPT for improving quality of life in schizophrenia and psychotic disorders, supported by the WELLFOCUS-PPT trial.
- **Chapter 13** applies PPT's concepts of balance, conflict, and capabilities to eating disorder treatment within interdisciplinary teams.
- **Chapter 14** bridges Posttraumatic Growth with the PPT Balance Model, offering case examples and the use of storytelling.
- **Chapter 15** addresses Somatic Symptom and Related Disorders, applying PPT concepts such as the Psychosomatic Arc.
- **Chapter 16** demonstrates PPT's conflict and balance models for use in child and adolescent therapy in individual, group, and family settings.
- **Chapter 17** shares insights from Polish colleagues on using PPT with individuals on the autism spectrum.
- **Chapter 18** highlights German colleagues' experience applying PPT to chronic pain syndromes.
- **Chapter 19** emphasizes the importance of clinician-patient communication using PPT frameworks to enhance therapeutic relationships.

Part IV: Special Settings and Populations

This section addresses diverse populations and innovative mental health practices through nine chapters.

- **Chapter 20** explores positive mental health among minorities, focusing on African American and Native American experiences and the importance of culturally relevant interventions.
- **Chapter 21** applies PPT's primary and secondary capabilities to diverse cultural contexts, clarifying family-of-origin concepts.
- **Chapter 22** examines PPIs in sports psychology to support athletes' mental health and performance.
- **Chapter 23** applies PPT tools to families and couples, including the Balance Model and Differentiation Analysis Inventory (DAI).

- **Chapter 24** demonstrates PPT's use in pedagogical settings to support teachers and students.
- **Chapter 25** applies PPT to group therapy, offering a case study involving individuals on the autism spectrum.
- **Chapter 26** integrates PPT and coaching in organizational settings using the OPTIC approach.
- **Chapter 27** explores challenges in engaging men in therapy and how PPT tools can address resistance.
- **Chapter 28** discusses the application of PPT to online psychotherapy, expanding global access to mental health care.
- **Chapter 29** deals with suicide intervention and prevention with Positive Psychotherapy.
- **Chapter 30** emphasizes the particularities of the therapeutic relationship when dealing with children, adolescents, and their families.
- **Chapter 31** shares the recent Ukrainian experiences regarding trauma counseling and crisis intervention during a war.
- **Chapter 32** explores the development of sexuality, as well as sexual education and counseling, through the lens of positive psychotherapy.

Part V: Theoretical Foundations and Training

This section includes ten chapters on foundational theories and advanced training techniques in positive psychotherapy.

- **Chapter 33** revisits PPT's historical and theoretical framework, connecting its roots to humanistic, psychodynamic, and cognitive theories.
- **Chapter 34** details the First Interview Process in PPT, including the Differentiation Analytic Inventory.
- **Chapter 35** explains PPT's four conflict types with a clinical case example of conflict resolution.
- **Chapter 36** explores storytelling in therapy, including traditional tales, humor, and movies.
- **Chapter 37** examines the therapist-supervisee relationship through the lens of PPT.
- **Chapter 38** highlights shared values in religious and spiritual traditions relevant to positive mental health.
- **Chapter 39** aligns PPT with existential philosophy, emphasizing meaning, responsibility, and balance.
- **Chapter 40** bridges PPT with other therapeutic modalities, exploring their intersections.
- **Chapter 41** underscores the importance of meaning in life as a cornerstone of PPT.
- **Chapter 42** highlights positive interpretation as a new way of looking at challenges and disorders.

Part VI: Emerging Topics and Contemporary Issues

This section addresses global challenges and innovative topics across 12 chapters.

- **Chapter 43** connects the therapeutic role of humor with neuroscience and the PPT Balance Model.
- **Chapter 44** examines the mental health implications of climate change and strategies for collective action.
- **Chapter 45** reflects on lessons from the COVID-19 pandemic in China, offering universal applications.
- **Chapter 46** discusses supporting LGBTQ+ clients using PPT to foster acceptance and growth.
- **Chapter 47** shares Ethiopian experiences with PPT in addressing war trauma.
- **Chapter 48** explores the challenges faced by refugee mental health workers assisting other refugees.
- **Chapter 49** highlights the experiences of Ukrainian therapists working with soldiers and families during the war.
- **Chapter 50** applies PPT to addressing the effects of sexual violence, with a focus on holistic care.
- **Chapter 51** examines multigenerational trauma in a Polish town through a PPT lens.
- **Chapter 52** shares insights from Ukrainian therapists on promoting resilience during wartime.
- **Chapter 53** discusses PPT techniques for helping immigrants adapt to new cultural contexts.
- **Chapter 54** explores applying PPT to personality disorders, offering a structured therapeutic model.
- **Chapter 55** describes the application of the balance model of positive psychotherapy in supervision of psychology students.

Acknowledgments

This book is the result of remarkable international and transcultural collaboration. The idea for this project originated during the 2016 Annual Meeting of the American Psychiatric Association (APA) in Atlanta, Georgia. From conception to the release of the first edition in 2020, it was a journey spanning more than three years—one that demanded a wealth of expertise and unwavering dedication from its authors.

In 2024, Springer Publishers approached us with the proposal to create a second, revised, and expanded edition. Recognizing the immense effort involved, we deliberated for a few days. Ultimately, our enthusiasm for producing a global textbook on Positive Mental Health prevailed.

This second edition features 55 chapters authored by 75 contributors from 19 countries across 5 continents. Its creation required extraordinary teamwork, cooperation, and perseverance. We are deeply grateful to the authors who generously devoted their time and expertise to writing original, innovative chapters without financial compensation. Their collective efforts have brought this comprehensive global textbook to life.

We would also like to extend our heartfelt gratitude to three esteemed professors whose inspiration and support were pivotal throughout this journey:

- **Professor Shridhar Sharma**, a pioneer of psychiatry and psychotherapy in India, facilitated connections with colleagues during the 2016 APA meeting. This led to the first-ever joint symposium on "Positive Psychiatry, Positive Psychotherapy, and Positive Psychology" in 2017.
- **Professor Dilip Jeste**, the founder of positive psychiatry, championed the idea of this book from its inception and graciously authored the Foreword. We are especially thankful for his leadership in chairing the aforementioned symposium and fostering a strong collaboration between himself, the World Federation for Psychotherapy, and the World Association for Positive and Transcultural Psychotherapy.
- **Professor Rama Rao Gogineni**, Cooper Medical School of Rowan University/Cooper University Hospital, embraced this project with enthusiasm. He introduced the concept to Springer Publishers and assisted in identifying contributing authors.

Finally, we extend our sincere appreciation to our publishing team: Ms. Julia Andrews, Clinical Medicine Editor, and Mr. Sharath Rangaraj, Production Editor. Their unwavering support and guidance ensured the successful completion of this edition. Despite challenges such as coordinating across time zones and cultural and linguistic differences, their efforts kept the project on track.

This book is a testament to the power of global collaboration and shared commitment to advancing the field of Positive Mental Health.

Saint Louis, MO, USA — Erick Messias
Wiesbaden, Germany — Hamid Peseschkian
May 2025

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Part I

Basic Concepts, Background, and History

Positive Psychiatry: An Introduction

1

Erick Messias

Introduction

Mental disorders are among the leading causes of disability worldwide [1]. Those millions of people living with mental disorders want more than finding a medication to alleviate their symptoms; they want to flourish and find satisfaction and happiness, like all human beings. For its first 200 years, the medical specialty of psychiatry has focused on the first goal of symptom control with significant successes along the way [2]. With the evolution of medicine and psychology, preventive and health-enhancing interventions are expected to be developed and implemented to go beyond treating disease and toward maintaining health and well-being. The same is happening in psychiatry with the development of Positive Psychiatry combining the tools of positive psychotherapy and the theoretical framework of positive psychology.

While psychologist Martin Seligman's 1998 presidential address for the American Psychological Association is known for launching the modern positive psychology movement [3], psychiatrist Dilip Jeste's 2012 presidential address to the American Psychiatric Association did the same to Positive Psychiatry [4]. In Jeste's address he urged psychiatry to aim beyond managing symptoms and identifying psychopathology to create tools that will help "patients grow, flourish, develop, and be more satisfied with their lives". These factors overlap significantly with those cited by Seligman 14 years before that included: optimism, courage, work ethic, future-mindedness, interpersonal skill, the capacity for pleasure and insight, and social responsibility.

A third contribution in the development of these approaches preceded these initiatives by a few decades, when positive psychotherapy was proposed by psychiatrist Nossrat Peseschkian as a "new treatment for psychiatric and psychosomatic illnesses" in Germany [5].

These three sources may be articulated in a more concise definition of *Positive Psychiatry as the application of Positive Psychology principles and tools, which include Positive Psychotherapy, to those with mental disorders or at risk of developing them.*

Definitions

Positive Psychiatry can also be described as the science and practice of psychiatry seeking to understand and promote well-being, through assessments and interventions aimed at enhancing Positive Psychosocial Factors (PPSFs) among those who have, or are at risk of develop-

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_1

ing mental or physical illnesses [6]. PPSFs include psychological traits and environmental factors—see below. *Positive Psychiatry has four main goals*: achievement of positive mental health outcomes, fostering PPSF, study of biological factors underpinning positive mental health, and development of positive psychiatry interventions [6]. These definitions of Positive Psychiatry contrast with the classic definition of the specialty, in which clinical psychiatry has been focused on symptoms, disability, diagnosis, and treatment—see Table 1.1. Looking through a lifespan perspective, clinical and positive psychiatry offer different approaches addressing different issues and problems—see Table 1.2.

Positive Psychosocial Factors (PPSFs) include psychological traits and environmental factors that mediate positive outcomes such as well-being, growth, and flourishing. The work of Seligman and Petersen validated a group of six virtues containing a total of 24 character strengths [7]. Their research looked into virtue traditions throughout history, including Confucian, Taoist, Buddhist, Hindu, Athenian, Judeo-Christian, and Islamic cultures. The resulting list of six major virtues includes: wisdom, courage, humanity, justice, temperance, and transcendence. The character strengths components of each virtue are listed in Table 1.3. In order to be included in this list, a character strength had to fulfill the majority of the following criteria:

- Criterion 1: a strength contributes to various fulfillments that constitute a good life, for oneself and for others. Although the strengths and virtues determine how an individual copes with adversity, the focus is on how they fulfill an individual.
- Criterion 2: although the strengths can and do produce desirable outcomes, each strength is morally valiant in his all right, even in the absence of obvious beneficial outcomes.
- Criterion 3: the display of strength by one person does not diminish other people in the vicinity.
- Criterion 4: being able to phrase the opposite of a punitive strength in a felicitous way

Table 1.1 Contrasting clinical and positive psychiatry perspectives

	Clinical psychiatry	Positive psychiatry
Target population	Those with mental disorders and behavioral symptoms	Those with, and at risk for, mental disorders
Immediate goals	Cure, management of symptoms, and return to baseline functioning	Improvement above baseline level of functioning
Long-term goals	Relapse prevention	Recovery in mental illness
Treatment approaches	Psychotherapy and psychopharmacology	Positive psychotherapy, executive coaching
Focus of attention	Symptoms, dysfunctions	Strengths, capabilities, talent
Theoretical foundation	Psychopathology and phenomenology	Positive psychology and resilience

Table 1.2 Contrasting clinical and positive psychiatry across the lifespan

	Clinical psychiatry	Positive psychiatry
Children	Reducing disruptive behaviors	Enhancing learning and development
Teenagers	Early detection of symptoms and early intervention	Promoting healthy coping skills and growth
Young adults	Relapse prevention and return to baseline functioning	Career development, promoting social engagement
Mid life	Maintenance of baseline, independent living skills	Overcoming baseline, achieving full potential
Elderly	Early detection and intervention, symptom management	Successful aging strategies

Table 1.3 Virtues and their components according to Peterson and Seligman

Virtue	Character strengths components	Definition
Wisdom	Creativity	Thinking of novel and productive ways to conceptualize and do things
	Curiosity	Taking an interest in an ongoing experience for its own sake
	Open-mindedness	Thinking through and examining things from all sides
	Love of learning	Having positive feelings about acquiring skills, satisfying curiosity, building on existing knowledge, and learning something new
	Perspective	Having ways of looking at the world that makes sense to oneself and to other people
Courage	Bravery	Not shrinking from threat, challenge, and difficulty
	Persistence	Capacity to stay the course despite obstacles
	Integrity	The quality of being honest and having strong moral principles
	Vitality	The state of being strong, energetic, and active
Humanity	Love	A great interest and pleasure in something
	Kindness	The quality of being friendly, generous, and considerate
	Social intelligence	Being aware of motives and feelings of other people and oneself
Justice	Citizenship	Working well as a member of a group or team
	Fairness	Impartial and just treatment or behavior without favoritism or discrimination
	Leadership	Organizing group activities and seen that they happen
Temperance	Forgiveness	Ability to stop feeling angry or resentful toward someone for an offense, flaw, or mistake
	Humility	Ability to acknowledge one's mistakes, imperfections, gaps in knowledge, and limitations
	Prudence	Cognitive orientation to the future in taking the form of practical reasoning and self-management
	Self-regulation	Optimal control over one's own thoughts, emotions, behaviors, and impulses
Transcendence	Appreciation of beauty	Ability to find, recognize, and take pleasure in the existence of goodness in the world
	Gratitude	Sense of thankfulness and joy in response to a gift or experience
	Hope	Positive cognitive and emotional stance towards the future
	Humor	Playful recognition and enjoyment of incongruity, including views on adversity, that leads to the ability to make others smile
	Spirituality	Beliefs and practices that recognize a transcendent—beyond ones' existence—dimension in life

counts against regarding it as a character strength.

- Criterion 5: a strength needs to be manifest in the range of an individual's behavior, including his or her thoughts, feelings, actions, and such a way that it can be assessed. It should be trait-like in the sense of having a degree of generality across situations and stability across time.
- Criterion 6: the strength is distinct from other positive traits in the classification and cannot be decomposed into them.
- Criterion 7: a character strength is embodied in consensual paragons.
- Criterion 8: there exist prodigies with respect to this strength.
- Criterion 9: there are people who show the total absence of a given strength.
- Criterion 10: the larger society provides institutions and associated rituals cultivating strengths and virtues and then for sustaining their practice.

Other positive psychological traits include resilience, optimism, personal mastery and coping self-efficacy, and social engagement [6].

Positive Translation of the conventional illness terminology has been proposed as a way to reframe disease and symptoms from deficits into capacities [8]. *Depression* can be positively translated from "feeling of being despondent,

with a prevailing passive attitude" to "ability to react to conflict with deep emotion". *Mania* can be interpreted not as "a mental illness" but also as "capacity to see the glass half-full, to experience one as powerful, and to disregard the minutiae of life". *Existential anxiety* moves from being "fear of the future" to being the "ability to prepare for the future and not give in the illusion of security" [8]. These interpretations are not necessarily more realistic but are ways to expand the view and experience of illness beyond a mere reductive dysfunction and handicap. This process of translation can also help link mental disorders to specific virtues or strengths of character.

Translating Mental Disorders to Strengths of Character

Mental disorders are associated with specific strengths of character, and this association holds promises of ways to translate these disease states into more positive capacities. Patients with Bipolar Disorder, for examples, have been known for their creativity and originality; patients with obsessive traits and symptoms are known for integrity and honesty; patients with depression can show prudence and humility; and finally persons dealing with mental disorders in general display significant elements of courage, like persistence and bravery, in facing their challenges and following up with treatment.

Recovery in Mental Illness has been defined as "a process of change through which individuals improve their health and wellness, live a self-directed life, and strive to reach their full potential" [9]. There are *four dimensions* that support life in recovery:

- Health—managing symptoms and diseases.
- Home, a stable and safe place to live.
- Purpose, daily activities that provide meaning and independence.
- Community, relationships, and social networks that provide support, friendship, love, and hope.

The ten guiding principles of recovery, as delineated by the Substance Abuse and Mental Health Services Administration (SAMHSA), are as follows:

1. Recovery emerges from hope.
2. Recovery is person-driven.
3. Recovery occurs via many pathways.
4. Recovery is holistic.
5. Recovery is supported by peers and allies.
6. Recovery is supported through relationships and social networks.
7. Recovery is culturally based and influenced.
8. Recovery is supported by addressing trauma.
9. Recovery involves individuals, family, and community strengths and responsibilities.
10. Recovery is based on respect.

Resilience is defined by the American Psychological Association (APA) as the process of adapting well in the face of adversity, trauma, tragedy, threats, or significant sources of stress—such as family and relationship problems, serious health problems or workplace and financial stressors. It means "bouncing back" from difficult experiences. Since having a mental disorder is one significant source of stress, the positive psychiatry perspective works to enhance and foster that trait. Among the ways to build resilience are: making connections, accepting change as part of living, taking action, using opportunities for self-discovery, and keeping things in perspective. Mental health promotion and human flourishing become then a natural outcome of resilience education and training [10].

Historical Background

Religious texts that are foundational to several civilizations contain the seeds of positive psychological traits and ways to foster them. As stated above, there is overlap on these traits across traditions and they form the basis of a universal theory of human strengths. At the source of Western—European-based—civilizations, in the works of Plato and Aristotle one finds extensive discussion on these virtues and character

strengths. In his monumental classificatory effort, Aristotle organized not only biological specimens but also worked on a theory of virtues.

Aristotle's Nicomachean Ethics

Through the ten books composing the *Nicomachean Ethics*, Aristotle builds his theory of virtues around the doctrine of the mean. In it, virtues occupy the space between extremes of behavior in that courage stands between cowardice and recklessness, confidence between timidity and arrogance, generosity between miserliness and vulgarity, and so on. With the fall of the Roman Empire, Aristotle's manuscripts were preserved in libraries through the Middle East, and returned to Europe in the 1200s when they were assimilated in Catholic theology by the hands of Thomas Aquinas.

With the development of clinical psychiatry in 1800's Europe most of its intellectual effort was dedicated to delineating conditions, establishing its border with neurology, and developing therapeutic interventions. In the mid-1900s American psychologist Abraham Maslow started a program to develop a theory of motivation initially based on five sets of goals in a hierarchy of needs [11]. Those five needs were: physiological, safety, love, esteem, and self-actualization. Maslow's work originated interest in human potential and motivation leading to interest in Creativity and Flow, as framed by Mihaly Csikszentmihalyi, the proposal of positive psychotherapy (PPT) by Nossrat Peseschkian, and eventually to Martin Seligman's push for the development of Positive Psychology.

Applications of Positive Psychiatry: First Episode Psychosis

Several principles and tools from positive psychiatry were used in the Early Treatment Program (ETP) study, part of the National Institute of Mental Health's (NIMH) Recovery After an Initial Schizophrenia Episode (RAISE) initiative. RAISE aimed to develop, test, and implement person-centered, integrated treatment approaches for first-episode psychosis that promote symptomatic and functional recovery. In line with these objectives, RAISE-ETP developed a comprehensive recovery-oriented, evidence-based intervention for first-episode psychosis that has been shown to be effective and statistically better than standard community-based treatment [12]. In summary, IRT involves weekly, or bi-weekly, structured sessions lasting between 45 and 60 min. The standard modules take 4–6 months to complete followed by individualized modules—both sets including sessions on resilience development. Among the individualized modules there are sessions specifically focused on "having fun", "connecting with people", and "improving relationships." A key message in the IRT process is that *recovery is possible in psychosis and that many persons with psychotic disorders live happy and productive lives*.

These applications of positive interventions in First Episode Psychosis corroborate the use of such perspective in what's traditionally known as "severe mental illness", or SMI, and will be further explored in the chapter on schizophrenia.

Applications of Positive Psychiatry: Post-traumatic Stress Disorder

The positive perspective and translation of post-trauma reaction beyond trauma to growth has been a key contribution in the development of positive psychiatry. Studies with the military have confirmed that negative responses to trauma, in particular the development of Post-Traumatic Stress Disorder (PTSD), are not the only possible outcome and that promoting post-traumatic growth may be beneficial for well-being, both in military populations [13] as well as civilians [14]. These elements will be further developed in the chapter on Trauma.

A positive psychiatry framework has been specifically developed to address PTSD among US veterans: cognitive-based compassion training (CBCT) [15]. CBCT is structured as an eight-session meditation protocol based on Tibetan Buddhist mind training and was first shown to decrease trauma symptoms with breast cancer

survivors and with children in foster care. Through these sessions participants increase their meditation practice time from 5 to 15 min while practicing exercises to assist in interpersonal growth and trust.

Applications of Positive Psychiatry: Depression and Anxiety

Positive interventions, including gratitude, acts of kindness, and optimism, have been shown to decrease negative affect, anxiety, and depressive symptoms in persons showing clinically impairing symptoms [16]. These interventions appear to have potential to not only decrease depressive and anxiety symptoms but also to increase well-being in clinical populations [17]. Finally, a meta-analysis of 51 positive psychology interventions concluded that they both enhance well-being and also decrease depressive symptoms [18]. Specific application of positive interventions in depression and anxiety is discussed in their respective chapters.

Applications of Positive Psychiatry: Child Psychiatry

Child Psychiatrist David Rettew [19] presents a wellness plan for a young child with behavioral problems. The wellness plan included interventions addressing the following:

Physical activity and exercise
Music and arts
Mindfulness
Monitoring screen time and content
Sleep
Nutrition
Charity—community engagement
Hydration
Positive parenting
Addressing parental psychopathology

As evident from this list, Positive Psychiatry goes much further than traditional clinical psychiatry approaches in developing and attending to the many facets of life—both at the family and at the social level. These many areas of life mirror the quadrants of PPT's balance model—see Chap. 2.

International Developments in Positive Psychiatry

In the Netherlands, psychiatrist Frenk Peeters and psychologist Fredrike Bannink have applied ideas and principles of Positive Psychiatry to their practice and developed a number of applications alongside a collection of stories for use by clinicians [20]. Along the same lines in Brazil, psychiatrist Leonardo Machado and psychologist Lina Matsumoto applied those principles to the local practices and initiated training in these perspectives [21].

Future Directions in Research in Positive Psychiatry

Research in Positive Psychiatry will mirror and expand the research portfolio in neurosciences. As such, the biological underpinnings, including genetics and neuroimaging, of positive psychological traits among those with mental disorder remain to be delineated. Initial efforts in understanding the neurobiology of wisdom show great promise in that approach [22]. The role of positive environment and social support in buffering life changes and disease stages also remains to be determined [23].

The study of effectiveness of positive interventions for specific diagnostic groups will also be needed, with calls for their application to those with psychotic disorders [24], PTSD [15], and depression [25] already on the way. The expansion of positive interventions toward addressing physical health needs and conditions is another area where research is active and promising [26–28].

Finally, application of Positive Psychiatry principles in the prevention and early detection of mental disorders is a future step of this process, when we move from individual-based interventions toward population health promotion.

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Positive Psychotherapy: An Introduction

2

Hamid Peseschkian and Arno Remmers

Introduction and Summary

Give a Man a Fish, and You Feed Him for a Day
Teach a Man to Fish, and He Will Feed Himself for a Lifetime

Positive Psychotherapy (PPT)[1] is a form of *humanistic psychodynamic psychotherapy* developed by Nossrat Peseschkian (1933–2010) during the 1970s and 1980s [1–3]. This method integrates approaches from the main modalities of psychotherapy: a humanistic conception of human nature and therapeutic alliance; a psychodynamic understanding of disorders; a systemic approach toward culture, work, and environment; and a practical, self-help, and goal-oriented five-step process of therapy in which techniques of different other methods can be integrated. As a conflict-centered and resource-oriented short-term psychotherapy method, it is based on transcultural observations in more than 20 cultures, which subsequently led to its development.

From the beginning, the *intention* of Nossrat Peseschkian was *twofold*: to develop an understandable, easily applicable, conflict-centered short-term therapy that professionals (psychotherapists, physicians, and psychologists) could use in their daily clinical work, to make psychotherapeutic and psychosomatic concepts accessible to interested laypersons, and at the same time provide them with instruments for self-help. Stimulated by his own transcultural life situation (Iran–Germany), Peseschkian was inspired to develop a method that could be applied without distinctions of culture or class.

Positive Psychotherapy pursues *several goals and can be applied in the following areas*:

- Psychotherapy and treatment of psychosomatic and mental disorders (therapeutic approach).
- Counseling, prevention, and education (preventive-pedagogic approach).
- Promotion of transcultural understanding (transcultural-societal approach).
- Cooperation and integration of different therapeutic methods (interdisciplinary approach).

The term "positive" in Positive Psychotherapy means that the aim of this therapy is not primarily

[1]Positive Psychotherapy (PPT after Peseschkian, since 1977) is a registered trademark in the European Union and the United States of America. Other registrations are being processed.

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_2

to eliminate an existing disturbance but rather to try at the outset to mobilize existing capacities and the potential for self-help. Rather than with the disturbance, PPT begins with the possibilities for development and the capacities of the people involved [3, pp. 1–7], following Maslow [4] who first used the term "positive psychology,"[2] as he saw "...the importance of focusing on the positive qualities in people, as opposed to treating them as a 'bag of symptoms'" [5] in humanistic psychology. Symptoms and disorders are seen by Peseschkian as a capacity to react to conflicts, and the therapy is called "positive" because it proceeds from the concept of the wholeness of the persons involved as a given. The pathogenesis of illness, conflict, deficit, and suffering is part of that wholeness as well as the salutogenesis with the joys, capacities, resources, potentials, and possibilities of those involved [6, p. 13].

Here, the term "positive" is to be understood in the sense of the "positive sciences" (based on Max Weber, 1988) as the judgment-free description of that which is observed. The Latin term "positum" or "positivus" is derived from the Latin "ponere," meaning "to place, put, lay down" (Merriam-Webster Online). Nossrat Peseschkian uses the above-mentioned term "positum" in a broader sense as an expression of that which is available, the given, the actual. This positive meaning confronts the patient (and the therapist) with a lesser-known aspect of the illness, but one that is just as important for the understanding and clinical treatment of the affliction: the function of the illness, its meaning, and consequently a connotation as given and positive aspects [7].

Based on a positive conception of human nature, *the concept of "capacity"*[3] runs throughout this method, such as viewing an illness as the expression of the capacity to react to a conflict, for example, as the capacity to be unique; as the capacity either to return to health or to be overcome by this illness; or as the specific capacities that are hidden in the conflict. A feature of this method is that the concepts that are used as well as the explanatory models can be easily understood by all patients.

Positive Psychotherapy fulfills the criteria of both humanistic psychotherapy and psychodynamic psychotherapy. We consider Positive Psychotherapy a psychodynamic method that is based on a humanistic conception of human nature. In this sense, the method can be described as a *humanistic psychodynamic psychotherapy* [8, pp. 13–15].

Brief History of the Origin of Positive Psychotherapy

> During the time that the Prophet Mohammed was born, King Anoshirvan, whom the people also called "The Just," traveled through his kingdom. On a sunny slope, he saw a venerable old man bent over, hard at work. Followed by his courtiers, the king came nearer and saw that the old man was planting year-old seedlings. "What are you doing there?" the king inquired. "I'm planting nut trees," replied the man. The king asked in amazement, "You are already quite old. Why are you planting seedlings when you won't see their foliage, won't rest in their shade, and won't eat their fruit?" The old man looked up and said, "Those who came before us planted, and we were able to harvest. Now we plant so that they who come after us can harvest." [9, p. 151]

The concept of PPT is closely connected to the life history and the personality of its founder Nossrat Peseschkian, an Iranian-born, German-trained neurologist, psychiatrist, and psychotherapist. His biographer referred to him as a "wanderer between two worlds" [10, p. 17]. It was not without reason that Peseschkian's biography was subtitled, "The East and the West."

Nossrat Peseschkian describes the origins of his method as follows: *"An important motivation for the employment of Positive Psychotherapy may have been that I find myself in a transcultural situation. As a Persian (Iranian), I have lived in Europe since 1954. In this situation, it was very clear to me that many forms of behavior, customs*

[2]Nossrat Peseschkian mentions the term "positive psychology" in his book on Positive Psychotherapy in 1987, p. 389, but not going further.

[3]Similar to Kurt Goldstein (1939), who saw self-actualization as "the tendency to actualize, as much as possible, [the organism's] individual capacities."

and attitudes did not have the same value in the two cultures. This was a discovery that I could already make during my childhood in Tehran. This concerned religious customs above all else, something which I could observe quite clearly. As Bahá'ís, we were always in the middle between our Moslem, Christian and Jewish classmates and teachers. This stimulated me to reflect upon the connections the religions have with one another and the relationships of people to one another. I had experience with the families of my classmates and learned to understand their attitudes as coming from their worldviews and family concepts. Later I was to witness similar confrontations during my medical specialization showing how tense the relationship was between psychiatric, neurological and psychotherapeutic positions and with what vehemence the psychiatrists and psychotherapists collided with one another. I learned that these prejudices must be discarded. This way I could also feel comfortable in the West. The equality of men and women, for example, was and still is a given for me" [10, pp. 62–63], translated by the authors from the German original.

The origin of PPT traces back to the development of humanistic psychology and psychotherapy, pioneered by Kurt Goldstein, Abraham Maslow, and Carl Rogers. Personal encounters with leading psychotherapists, such as the psychoanalyst Heinrich Meng (Switzerland), Viktor Frankl (Austria) and his existential psychology or Logotherapy, Jacob Levi Moreno (United States) as the founder of group therapy and psychodrama, and training experiences with some of them left a lasting impression on Nossrat Peseschkian. As for the quarrels between the various schools and methods, Peseschkian describes his observations in Germany, where psychoanalysts and behavior therapists sometimes even refused to have lunch together. Additionally, the strong prevailing influence of psychoanalysis and its Neo-Freudian, psychosomatic, and focus orientated (Balint) developments at that time also made an impression on Peseschkian. He hoped to construct a metatheory that would be a bridge between these methods.

At the same time, some principles of the Bahá'í Faith, such as the harmony between science and religion [11–13], the Bahá'í conception of human nature as *"a mine rich in gems of inestimable value"* [14, CXXII, pp. 259–260], and the vision of a global society united in diversity, fascinated and inspired Peseschkian throughout his life. Continuing medical education; experience with patients in a psychotherapeutic and psychosomatic practice; contacts with people from different cultures, religions, and value systems; and the diversity and heterogeneity of the methods of psychotherapy were the sources of what gradually developed as "Differentiation Analysis" from 1969 on, presented at conferences, into Positive Psychotherapy in 1977.

The title of the first book "Psychotherapy of Everyday Life" [15] (in 1974), and of the book "In Search of Meaning" [16] (in 1983) show the influence of psychoanalysis and the existential schools of psychotherapy on the development of Positive Psychotherapy,[4] and the title "Positive Family Therapy" (in 1980) shows the common development with systemic family therapy in the 1970s. There are 29 books and hundreds of articles on this method by the founder himself.

Main Characteristics of Positive Psychotherapy

The application of PPT as an integrative, transcultural, and humanistic psychodynamic psychotherapy will be explained in many chapters of this book. In this chapter, the main principles and characteristics are explained as follows:

Positive Psychotherapy as a Metatheory

From the beginning, Nossrat Peseschkian's purpose was first, to develop a method that would be understandable and practicable for patients, and

[4]Some North American authors have published the clinical applications of positive psychology and named it also Positive Psychotherapy (Martin E. P. Seligman, Tayyab Rashid, Acacia C. Parks, Positive Psychotherapy. November 2006, American Psychologist, 774–788) [17].

second, to offer PPT as an agent of mediation among the different schools of psychotherapy. In his early book "Positive Psychotherapy" (1977 in German, 1987 in English), he devotes an entire chapter to this challenge: "Positive Psychotherapy and Other Psychotherapies" (pp. 365–400). He often referred to that chapter as the most difficult one in the book and the one that required the most work.

> Positive Psychotherapy is not to be understood as one method among others. Rather, it offers an instrumentarium by means of which one can select the methodological approaches appropriate in a specific case and how these methods can be alternated. Positive Psychotherapy is thus a metatheory of psychotherapies. We understand psychotherapy to be not just an established method to be applied to certain symptom profiles, but at the same time as a reaction to existing societal, transcultural, and social conditions. [3, p. 372]

> Positive Psychotherapy itself is not to be understood as an exclusive system, but rather attributes a particular value to each of the various psychotherapeutic methods. Thus, psychoanalytic, depth psychological [psychodynamic], behavior therapeutic, group therapeutic, hypnotherapeutic, medicamentous, and physiotherapeutic forms of treatment are considered. Positive Psychotherapy thus represents an integral method, in the sense of a multidimensional therapy. [3, p. 400]

Nearly 20 years were to pass before Klaus Grawe and his colleagues (Switzerland) published a meta-analysis about the effectiveness of the different approaches and founded a general method that would go beyond the schools of psychotherapy [18]. In the United States, Jerome Frank published a scheme for integrated psychotherapy [19], but this plan was also met with controversy and was not accepted. The movements for eclectic and integrative psychotherapy, which have found increasing acceptance since that time, have nonetheless skirted the core goal of theoretical integration and largely settled for the peripheral function of employing techniques from various schools [20, 21]. Today there is an agreement that the common factors and the influence of the therapeutic alliance, empathy, expectations, cultural adaptation, and the person of the therapists are more important than the methods and techniques as such [22].

The Transcultural Approach

The inclusion of a transcultural viewpoint in the everyday work of psychotherapy was not merely a central concern of Peseschkian from the very beginning; rather, transcultural questions had a sociopolitical dimension for him: *"The transcultural approach runs like a red thread through the whole of Positive Psychotherapy. We consider it especially because the transcultural point of view also offers material useful for the understanding of individual conflicts. Furthermore, this point of view possesses extraordinary social significance: Problems of guest workers* [immigrants], *of help with development, problems which arise in dealing with people from other cultural systems, problems of transcultural marriages, prejudices and overcoming them, alternative models which originate from another cultural framework. In this connection we can also address political problems which originate in a transcultural situation"* [3].

The consideration of cultural factors and the unique characteristics of every treatment leads not only to a broadened area of application for PPT but also to its effective use in multicultural societies [23]. PPT has been taught and is applied by psychotherapists in some 60 countries, and it can be understood as a transcultural method of psychotherapy. Consequently, the principles of PPT serve as the basis of the definition and construction of the new scientific discipline of transcultural psychotherapy, particularly important for psychotherapy education, for continuing education, and for the recognition and implementation of new disciplines in psychotherapy.

The term "transcultural" in PPT means (a) the consideration of the uniqueness of the patient when the therapist and the patient come from different cultural backgrounds (intercultural psychotherapy, or "migrant psychotherapy"), and (b) the consideration of cultural factors in every therapeutic relationship in the sense of broadening one's own repertoire, as a therapeutic attitude, and consequently a sociopolitical dimension of our thinking and behavior. This consideration of the uniqueness of each person, of the relativity of human behavior, and of "unity in diversity" is an

essential reason why PPT cannot be characterized as a "Western" method in the sense of "psychological colonization" [24]. Rather, this approach is a culture-sensitive method that can be modified to adapt to a particular culture and life situation. Referring to this assertion of the social dimension of PPT, Nossrat Peseschkian writes:

> When we carry over these considerations into the whole realm of social relationships, including the interrelationships among groups, peoples, nations, and cultural groups, a bold social theory may be developed in accordance with Positive Psychotherapy, a theory which places great emphasis upon both difficulties in interaction and the human capacities as well as upon economic conditions. [3, p. 7]

In this sense, transcultural psychotherapy must be understood as an all-embracing concept and not merely as a comparison between different cultures. This form of psychotherapy is essentially a matter of the cultural dimensions of human behavior, asking: *How are people different? What do all people have in common?* [3, p. 4]. Furthermore, examples from other cultures are given to the patient to help him or her relativize his or her own perspective and to broaden his or her own repertoire of behavior. In PPT, the use of stories and tales, social norms (actual capacities), and the Balance Model are helpful towards a transcultural perspective. In 1979, Peseschkian used the term "transcultural psychotherapy" to which he dedicated a chapter in his work "The Merchant and the Parrot: Oriental Stories in Positive Psychotherapy": "*Transcultural difficulties—in private life, work, and politics—are growing increasingly important today. Given the way society is developing now, the solution of transcultural problems will create one of the major tasks of the future"* [9, p. 21]. He explains further that the principle of transcultural problems thereby becomes the principle of relationships between people and of dealing with inner spiritual conflicts, finally becoming the object of psychotherapy.

The Use of Stories, Tales, Proverbs, and Anecdotes

A special technique of PPT is the therapeutic use of tales, stories, and proverbs, which was introduced in the earliest works of Nossrat Peseschkian, such as "Oriental Stories as Tools in Psychotherapy—The Merchant and the Parrot" (in 1979). The therapeutic effect of surprise produced by these Eastern stories, which at first seem "strange" in European culture, is clearly intended. The use of unfamiliar stories has also proven effective in other cultures.

Because of the models that stories present, they can have *various functions* in therapy [9, pp. 24–34]. On one hand, stories create norms against which readers or listeners can measure themselves; on the other hand, they pointedly call norms into question and invite people to view them as relative. In therapy, these stories can be the means of a change in point of view, which is the goal of the first stage of therapy and is then used in the other stages. Such narratives can free up the feelings and thoughts of the listeners and often are key moments in therapy. The mirror function of storytelling leads to identification. In the narratives, the reader or listener recognizes him or herself as well as his or her needs and situation. He or she can reflect on the stories without personally becoming the focus of these reflections, and he or she can remember his or her own experiences. Stories present solutions that can be models against which one's own approach can be compared but that also leave room for broader interpretation. Storytelling is particularly useful in bringing about change in patients who are holding fast to old and outworn ideas.

The long-term effect of these narratives can also be seen in therapy. Many patients do not react to them at first. The authors have known many people who have come back to a story sometime after they had first reflected upon it, producing a "prolonged effect." Storytelling also aids rational people in regression. Patients recall their unconscious childhood encounters inspired

by pictures, fairy tales, and stories. Passed down from one generation to the next, fairy tales and other stories and narratives have always served as transmitters of culture and tradition. Even when repeated many times, stories always acquire a new meaning for each listener. In PPT, the patient is asked about favorite fairy tales and identification with their figures or actions. The answers often give clues regarding the basic concepts or conflicts that are at work.

The stories function as transcultural communication by mirroring the behavior and thinking of people in other cultural circles. The fascination with the foreign causes the person's own rules to be viewed relatively. Some stories have the effect of direct provocation: they present an alternative concept that the patient must come to terms with. Above all, stories belong to the fourth stage, with its conflict-laden verbalization and confrontation. Laughing over a story in therapy changes the view, unties the "neurotic" knots. *"Humor is like salt in the soup of therapy"* (Nossrat Peseschkian, personal communication); it opens up insight and relativizes fixed concepts. In psychodynamic therapy, humor is regarded as one of the most important, effective means of overcoming challenges, even in the most difficult and most tense situations [25, p. 48].

Resistance in therapy can also be understood as a capacity to hold onto the past and to oppose change. Stories and proverbs are among the most effective means of overcoming resistance. The triangle of the patient, the therapist, and the story provide an additional dimension of transference to explore. The characteristic of a psychodynamic method is shown clearly here: stories are aids in association that lead the way to the deeper, unconscious core. Early experiences with stories that were told by the primary educators during the so-called "magic phase" of 3–6 years of age are significant in the understanding of the relationship between good and evil. These experiences build the basis for the development of conscience. The stories contain figures with whom the children can identify and whom they learn to trust, which help them in their own experiences. Children can understand stories told to them by close persons even when their objective understanding is not yet developed.

The Positive Conception of Human Nature and Positive Interpretation of Disorders

The concept of PPT is based on a humanistic conception of human nature[5] in which people are essentially good by nature and endowed with many capacities [26]. In PPT, one speaks about the two basic capacities: to love and to know (see Fig. 2.1). Within biological limitations, the development of a person's capacities will, through education, cause him or to develop a unique personality. In this sense, education is a lifelong process of maturation; in such a context, therapy can be viewed as presenting the possibility of further maturation and of broadening the education of the patient and his or her family.

From this positive perspective, illnesses can be seen as a means of developing and managing conflicts in a way that was often learned early in life. Disorders are then understood as one-sided reactions to the dilemma between available concepts and capacities that are insufficient and not yet developed. By broadening the concepts and developing the capacities into abilities, new internal models develop that broaden the former one-sided perceptions, thinking, expectations, and behavior into new possibilities.

Thus, *disorders are interpreted in a positive way*. Depression, for example, can be seen as "the ability to react to conflicts with deep emotionality"; fear of loneliness is understood as "the desire to be with other people"; alcoholism becomes "the capacity to supply oneself with that warmth (and love) which is not received from others"; psychosis is viewed as "the capacity to live in two worlds at the same time"; and cardiac disorders become "the capacity to take something very close to one's heart" [7].

The positive process brings with it a change in perspective in all those concerned: the patient,

[5]In the German language, there is a very exact word for worldview, philosophy of life, or image or conception of human nature: *Menschenbild*. This concept plays a very important role in philosophy, medicine, psychiatry, and psychotherapy.

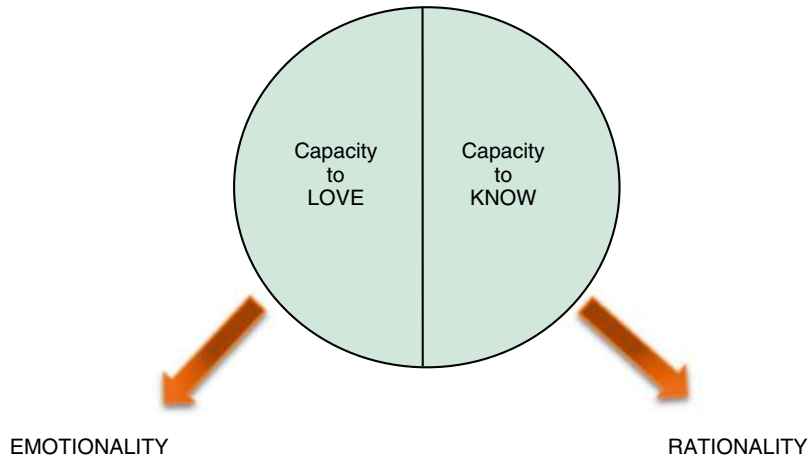


Fig. 2.1 The basic capacities in Positive Psychotherapy and their function

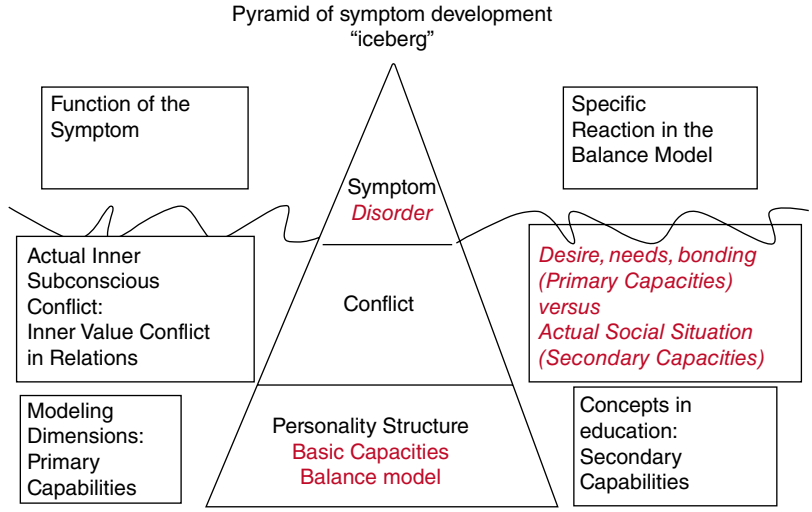


Fig. 2.2 The iceberg concept: from symptom to conflict

his or her family, and the therapist/physician. In this way, one moves from the symptom to the conflict (see Fig. 2.2). This process also helps us focus on the "true" patient, who often is not our patient. The patient who comes to us functions as a symptom carrier and can be seen as the "weakest link" in the family chain. The "real patient" is often sitting at home [27]. The positive interpretation of illnesses confronts the patient with the possible function and the psychodynamic meaning of his illness for him- or herself and his or her social milieu, encourages the patient (and his or her family) to see his or her abilities and not merely the pathological aspects.

The Concept of Balance

Based on the humanistic conception of human nature and the resources every patient or client brings with himself or herself, a core concept of PPT is the importance of balance in one's life. Nossrat Peseschkian's concept of the Balance Model (see Fig. 2.3) [28] [other terms being the "four areas, qualities or dimensions of life"] is known as the very core of Positive Psychotherapy and is applied in different settings—clinical and nonclinical.

The Balance Model is based on the concept that there are essentially four areas of life in which a human being lives and functions. These

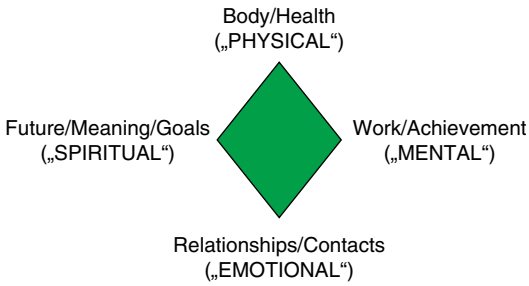


Fig. 2.3 The balance model of positive psychotherapy

areas influence one's satisfaction in life, one's feelings of self-worth, and the way one deals with conflicts and challenges and are the hallmarks of one's personality in the here and now. This model describes and connects the biological-physical, rational-intellectual, socio-emotional, and imaginative-spiritual spheres and capacities of human beings in everyday life. Although the potential for all four areas and capacities is present in every human being, some will be especially emphasized and others neglected through differences in education and environment. Life energies, activities, and reactions belong to these four areas of life:

1. Physical activities and perceptions, such as eating, drinking, tenderness, sexuality, sleep, relaxation, sports, appearance, and clothing.
2. Professional achievement and capabilities, such as a trade, household duties, gardening, basic and advanced education, and money management.
3. Relationships and contact styles with partners, family, friends, acquaintances and strangers; social engagements and activities.
4. Future plans; religious/spiritual practices; purpose/meaning; meditation; reflection; death; beliefs; ideas; and development of vision or imagination-fantasy.

The goal is to recover balance among the four areas. A goal of psychotherapeutic treatment is to help the patient recognize his or her own resources and to mobilize them with the goal of bringing them into a dynamic equilibrium. In particular, this goal places value on a balanced distribution of energy (25% to each area), not of time. One-sidedness that continues for too long can lead to conflicts and illnesses, among other things.

The positive conception of human nature becomes particularly obvious when seen in the interpretation of an individual's life balance. Instead of pointing out a deficit to the patient or those close to him or her and advising the patient to lower his or her ambitions, the positive aspects of the one-sidedness are emphasized. The patient is encouraged, and his or her weak feeling of self-worth is strengthened to create a basis for analysis of the areas with the deficits.

For example, a person who places a high value on achievement and so works long hours every day is not confronted at an early stage with the fact that he or she should spend more time with family. At the outset, his or her inclination for achievement and motivation to work are observed and identified as a capacity. This experience is constructive for the patient and is both important and fruitful for building the relationship between the therapist and the patient. At the same time, the focus goes away from sickness or family problems to neglected other areas of life to develop them—a practical application of salutogenic principles [29].

At the same time, the Balance Model embodies the four spheres of life that potentially give self-esteem to a person. Usually, only one or two areas give self-esteem, but a patient can learn in the therapeutic process to uncover the neglected areas so that his or her self-esteem will have additional pillars of support.

The Conflict Model of Positive Psychotherapy

Nossrat Peseschkian's psychodynamic conflict model (see Fig. 2.4) presents the differentiation of the content that is the point of contention and its internal evaluation. This model differentiates between the *actual conflict* that appears in an overburdened situation, the pre-existing *basic conflict,* and the unconscious *inner conflict* which causes physical and/or mental symptoms. "Conflict" (confligere, Lat., clash, fight) presents

Fig. 2.4 The concept of the three main conflicts in PPT

BC + AC → AISC

Basic Conflict	Actual conflict	Actual Inner Subconscious Conflict
Former experience of environment, one self and interaction 	Actual situation, life event, mictrotraumatic situation	Subconsciously unbearable situation, decompensation of coping strategies
Psychic adaptation, compensation and defense mechanisms	Micro-or Macrotrauma 	Hopelessness
Development of personality and individual structure 	reactivates the sleeping Basic Conflict 	Conflict manifestation symbolically in body or psyche 

the apparent incompatibility of inner and outer values and concepts or an inner ambivalence. Emotions, affective states, and physical reactions can be understood in this context as signal lamps that indicate an inner conflict of values and the distribution of actual capacities. Therefore, in PPT, a question is asked about the content: what causes or triggers this emotion?

As an example of the translation of emotion into the content of a conflict, we can imagine the almost proverbial tube of toothpaste that is crushed against the glass while lying in the holder every morning or any similar everyday situation. Some such thing triggers a feeling of displeasure in one partner. Another person might react to the same situation with disappointment, tension, or even envy according to which conflict content is consciously or unconsciously significant to the person at hand. Is it the feeling of orderliness that is disturbed ("I will not have such disorder in my bathroom")? Is it a matter of thrift ("Now the tube cannot be squeezed to the end, and we must buy a new one before it is time")? It may be a perception of injustice ("Every day, I am upset that you people have it so good. Must I also clean up after you?"), or it may be a matter of diligence with the secret wish to leave everything as it is for once, but for the habit of doing everything ("I would like to have it so good that someone takes care of me"). There is always strained patience ("It's been a long time of no confidence and dashed hopes. We have talked about it so often, but nothing ever changes."). These are examples of subjective evaluations that have appeared so often in everyday life that they have come to function as unresolved inner conflicts that undermine the patient's health, just as *"constant dripping wears away the stone."*

The repetition of these many small psychic injuries, the "always" and "again!" with the microstress that is received from them, is what Nossrat Peseschkian calls **"microtraumas**," the so-called "trivia, or trifles" [3, p. 80], in contrast to single, major life events, which are *macrotraumas*. Peseschkian described these microtraumas, which bear conflict content, in terms of *actual capacities*. They facilitate and form relationships and, on the other hand, are poised to become conflict content under specific conditions. The capacities attain this function in an *actual conflict*, a triggering situation that places too much demand on the previously effective coping mechanisms. Such a situation wakes up an old, unconscious *basic conflict*, as the patient is trapped between his or her primary emotional needs on one side, which are represented by primary capacities, like trust, hope or tenderness, and evaluations on the other, which are described as secondary capacities or social norms like orderliness, punctuality, justice or openness. The

compromise that had worked out the basic conflict before no longer functions; an *inner conflict* is the result, standing between two values in a dilemma. This evokes symptoms that can be viewed as attempts at a solution. The conflict reactions can be presented using the Balance Model. Although such reactions cannot bring about a solution, they still have influence.

> The actual conflict situation does not usually appear like a bolt out of the blue. In part, it develops very slowly, finally reaching an intensity whereupon what would otherwise be a less problematic conflict situation suddenly becomes a psychological or psychosomatic disturbance. To make this clearer: disturbances of the soul resemble water which is slowly heated and finally, when a certain temperature has been reached, begins to boil. For this reason, one looks beyond the actual conflict and investigates the course of development of the inward and outward conflict situation; in so doing, the person's childhood and the milieu in which he was brought up become the foci of psychotherapeutic inquiry. Here we come upon domains in which we are concerned with relatively constant, i.e. stable characteristics and attitudes, which are labeled personality traits. The conflict situation which appears at this level of analysis is called the basic conflict. In the basic conflict are evaluations and weightings, as well as the lion's share of individual development with regard to the modes of the capacities to know and to love. These are just as much prerequisites for basic conflicts as is the pattern of actual capacities acquired in the course of a person's lifetime. The basic conflict need not be a one-off event, such as the death of a reference person. Rather, all our experiences which we have had regarding the actual capacities and the modes of the basic capacities, and which play a part in the shaping of the actual conflict situation, are inputs into what we sum up as the basic conflict. [3, p. 129]

Disorders can occur when these capacities, virtues, concepts, or values are now repeatedly applied in a one-sided way without the modification appropriate to the present situation. The continuing repetition of family (basic) concepts that were appropriate at an earlier stage of development or of compromises made with them at an early stage (basic conflict) leads to an unconscious inner conflict. Psychic, psychosomatic, or bodily disturbances then occur and become the unconscious expression of the inner conflict. These manifestations fulfill an important function in the conflict by acting as the speech of the body or the spirit. Thus, the disorders have the function to help the client to express something subconsciously and have a specific meaning for the particular patient. PPT proceeds so that areas that have previously been neglected, capacities that have seldom been used and must be developed, are strengthened both within the therapeutic relationship and in the stress of the everyday environment to enable patients to resolve conflicts constructively and to restore inner and outer balance [30].

The Four Role Model Dimensions

One of the most important and challenging tasks in psychodynamic psychotherapy is the assessment of the effect of early childhood on a patient. One tool in PPT is the concept of the Four Role Model Dimensions (see Fig. 2.5). This model describes the pattern of family concepts in which the individual grows up in a way that reflects the individual's experience of them. One's upbringing and the environment during early childhood lead to a certain unique development and presentation of the basic capacities which Peseschkian called the capacity to love and the capacity to know. The means of the capacity to know are described in the section on the Balance Model; the means of the capacity to love will be described below.

Positive Psychotherapy proceeds from four typical basic relationships which every human being experiences or can experience in some

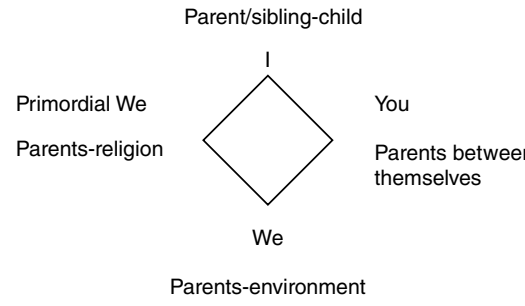


Fig. 2.5 The four role model dimensions of positive psychotherapy

form. We may also speak of them as four levels or types of emotional relationship. We distinguish among four modes or means of the capacity to love. They characterize typical basic relationships into which every person enters in one way or another. These basic relationships are the relationships to the I, to the You, to the We, and to the Primordial We. A characteristic model dimension influences the development of each of these relationships. The structure of these relationships can be asked about and presented graphically during therapy. Peseschkian has put questions for each of these spheres together which can be used in therapy and also given to a patient as some kind of homework until the next session [28].

Dimension "I"

The model dimension for the relationship to myself and life-long issues such as self-esteem, self-confidence, self-image, and basic trust versus basic distrust depend to a great extent on the relationship of my parents (and siblings) to me when I was a child. Here the child learns to assume a relationship to himself, which especially depends on how his wants and needs are satisfied. The basic question "Am I accepted or rejected?" is decisive. The answer to this question is provided directly by the reference person-child relationship, and indirectly by the comparison with the treatment of the siblings, for example. In the process a first discovery is made, which, to a certain degree, can become a reference system for later developments. It may be presumed that this initial discovery influences the later techniques of determination of self-worth: "Am I accepted on the grounds of my personality or on the grounds of my achievements?" Here lies the cause for the origin of basic conflicts. Later problems of self-esteem are essentially traced back to a conflictual, deficit-laden development in this area of the "I." This is also true for the capacity to bond and relate, early development of trust as opposed to mistrust, the basic attitude toward life, identity, and self-image as man or woman. Above all, the primary actual capacities are developed in this area in mutual interactions which demonstrate the modeling functions of the primary reference person. This is equivalent to the experience of the self-object [31] and the development of self- and object-representation in psychoanalytical theory [32]. The grandparents also come in here in their independent role as trans-generational transmitters of tradition and preferred primary capacities and in the widening triangulation they play a significant role in their grandchild's development of self-esteem because of their special form of acceptance of him or her.

Dimension "You"

"The preeminent model for a person's relationship to the "Thou/You" is the parents' example, i.e., the parents' relationship to one another. It is through the parents' example that the possible ways of behaving in a partnership are suggested. Most importantly, it is through it that behavioral forms of tenderness are imprinted" [28]. Concepts and conflicts such as fidelity, the capacities to have a relationship, to advise someone, intimacy, togetherness, give and take, the capacity to be or to choose a partner, the self-image as man or woman, or the capacity to conflict—all these have their origin mainly in the healthy or deficit-ridden development in this area. The three-cornered relationship of father, mother, and child has special influence in the identity as man or woman, the duality of love or justice or the perception of rivalry and solidarity [28]. This dimension plays an important role in couple therapy and/or marital problems.

Dimension "We"

A person's relationship to the "We" is modeled to a great degree by his parents' relation to their social surroundings. With socialization, characteristic attitudes toward social behavior are transferred, in addition to achievement norms. The attitudes and expectations are related to social ties outside of the nuclear family. They include the relation to relatives, colleagues, social refer-

ence groups, interest groups, compatriots, and also humankind as a whole. Here one develops less of a relation with a personal You and more relationships with social groups. As a social being, a person adheres to the group, which offers him or her a series of essential assurances, but which can be experienced as threatening, for through confrontation with others one's own values can be called into question. For some people, this is sufficient grounds for only seeking contact where they can count on assent, and where the same patterns of actual capacities and modes prevail. A group demands of its members recognition of certain norms which are conventional for the group, and thus, in some cases, a relinquishing of one's natural inclinations. Representative social groups comprising the "We" are institutions, i.e., social establishments such as societies, professional associations, churches, sport clubs, and political parties, and also psychotherapeutic groups, and in general all such establishments as make it possible for a person to speak of a "We" [28]. Concepts and conflicts such as the capacities to have social relationships, to be part of a group, to accept the conventions of the group, the need for contact, involvement in hospitality, need for emotionality, the need for peace, relationships with people from other backgrounds, withdrawal versus sociability—all these have their origin mainly in the healthy or deficit-ridden development in this area. Role models develop and group identity gives support when the rules of the group are accepted, the value of which have been predetermined in the area of the "We."

Dimension "Primordial We"

A person's relationship to the Primordial We depends, in the first instance, on his parents' attitude toward meaning, purpose, spirituality/religion, and worldview. Thus, the Primordial We is not only based on formal membership of a religious community but is also fundamental for the question of meaning which is asked later. Even if religion is rejected, the Primordial We remains the basis for other systems of orientation which are expected to provide meaningful content. As such systems we have the society (and more frequently a determined societal form), a certain way of life, the family, a chosen model, or the achievement or pleasure principle. These ideational contents can become an idol or a substitute religion. Even more determinant than the handed-down contents seem to be the conviction and consistency with which the parents defend those contents [28].

Concepts and conflicts such as doubt, hope, materialism, moral attitudes, fanaticism, crisis of faith, life after death, meaningfulness versus meaninglessness, these basic values have their origin mainly in the healthy or deficit-ridden development in this sphere. The parents' relationship to each other and the attitude of more distant relatives with regard to questions about the philosophy of life, for example, religious norms are also significant. The later valuation of actual capacities as moral norms, as motto or ideal, and with them the formation of conscience take their point of departure mainly from the "Primordial We."

A Semi-structured Approach

As a method that finds itself between manualized cognitive behavior therapy and process-oriented analytical psychotherapy, PPT pursuits a semi-structured approach in diagnostics (first interview), treatment, post-therapeutic self-help, and training (self-discovery).

The First Interview in Positive Psychotherapy

"Clinical interviewing is the single most important skill required in psychiatry" [33, p. 7]. The first interview in PPT holds a special place and represents a distinctive feature in the psychodynamic process. This semi-structured and half-standardized "First Interview of Positive Psychotherapy" [34] allows us to recognize the particular psychodynamic and biographical characteristics and resources important to the origin and development of the patient's state of health or

illness, and it also serves as a systematic starting point for brief therapy. To our knowledge, this first interview is one of the few existing in psychodynamic and humanistic psychotherapy (in Chap. 34, the first interview of PPT is published for the first time in the English language).

The psychotherapeutic first interview is central to the success of the therapy and is comparable to the medical history and the physical examination in somatic medicine. The first interview has a diagnostic, therapeutic, prognostic, and hypothesis-formulating function [35]. The first interview in PPT includes the diagnostic approach of the (psycho)somatic medical history as a means of inquiry. The interview considers relationship factors [36] as well as aspects of the therapeutic alliance [37]. The effects of expectation are recognized [36], particularly the hope for effective therapy [19, 38, pp. 193–212]. Above all else, because of the semi-structured nature of the interview and the concepts it employs, it is useful in many different settings, in therapy with individuals, children, youth, couples, and families, in counseling and coaching, and is practiced in different cultures.

The first interview in PPT consists of obligatory main questions and optional subordinate questions, which can be addressed or not, depending on the answers given to the main questions in each situation. There are open-ended as well as closed questions [34, p. 31]. We generally think of the first interview as being used during the first meeting or during the early sessions that form the preliminary phase, but it is possible to begin the first interview during the first session for the purposes of orientation and then go into greater depth in particular areas during the following sessions.

The Five Stages of Positive Psychotherapy

A structure of communication is employed within the *three stages of interaction* (attachment/connectedness, differentiation, detachment/separation), which involves a *5-stage process* that is used both within each session and throughout the therapy.

1. The first stage in the process is *observation and distancing,* which involves a change in point of view.
2. The second stage is the taking of an *inventory* in which the content of the conflict and the patient's strengths are *differentiated*.
3. In the third step, called *situational encouragement*, self-help and resources are developed.
4. The conflict is subsequently worked through in the fourth step, that of *verbalization.*
5. The fifth step, called *goal expansion,* includes a future-orientated reflecting on, summarizing, and testing the new concepts, strategies, and perspectives.

This therapeutic process is predominantly oriented toward the future and toward change, and it involves understanding the past through concepts that are effective for the present. In this way, these adapted concepts are used, as well as those from other disciplines of psychotherapy when appropriate (integrative experience). The patient and his or her surroundings play an active part in the understanding of the process of illness (self-help).

The 5 stages provide a structure for the process of communication within a single session, or during the whole therapeutic process, which would spontaneously come to an end without it. The therapist facilitates the stories by means of an appropriate attitude, leading questions, stories, association triggers, and revisiting themes previously mentioned. The 5-stage process in PPT provides both the therapist and the patient with a starting point as well as a sense of security and prepares the patient to work with conflicts and to engage in self-help, particularly after the end of therapy.

Stage 1: Observation and Distancing

The patient begins with an emotional supposition; a concrete inventory is made, and a new perspective is established. The patient is led from an abstract stage of suffering to a concrete, descriptive point of view. He or she is brought to an understanding of the functions and effects of his or her symptoms in the four areas of life, with the

use of figures of speech and transcultural comparisons. As part of the therapy, the patient is asked to observe the situations he experiences and his own emotions, particularly with conflicts, and to write them down as spontaneously as possible without changing anything. Viewing his own conflicts from the position of an observer helps the patient attain a growing distance from his own conflict situation. The patient becomes an observer of him- or herself and his or her environment. An important effect of this stage is the high level of unburdening in a conflict situation. Often, by the time a patient comes to therapy, he is "at the end of his rope," and therapy is "the last stop." It does no good to comfort the patient with the hope of successful therapy in 6–12 months. He is looking for help at the moment and for relief. In the same way, the stage of observation is helpful from the beginning of treatment in interpersonal relationships such as between life partners or in the workplace because the patient becomes much less critical and all conflicts and pain are avoided. Tense situations are defused, and time is gained. The patient is informed that he will be able to talk about everything during the fourth state (verbalization) and that he will prepare for this step during the next three stages, with his conflict partner if appropriate. The broadening of what is at first a one-sided perception, a "neurotic narrowness," provides the preparation for the next stage in the process, making an inventory of events.

Stage 2: Inventory (Inventory-Taking)

This stage introduces the paramount task: to recognize the connections to clarify the prehistory of the individual's actual capacities and the preparation for the conflict, as well as to put the background of the concepts and misunderstandings into order so that the patient can develop a means of understanding them for him- or herself. The positions (attitudes) that, as a rule, the patient sees as being unchangeable personality characteristics will now be viewed as relative according to their meaning in his or her life history. The significance of the actual capacities is summed through association by means of an inventory of the actual capacities (the Differentiation-Analytic Inventory, DAI). The Wiesbaden Inventory for Positive Psychotherapy and Family Therapy (WIPPF) questionnaire provides a complete summary of the areas of behavior in which the patient and, if appropriate, his partner or conflict partner possess positively or negatively evaluated characteristics and where there are strengths, microtraumas, or examples of conflict reaction. The WIPPF also facilitates access to role models.

Patients with psychosomatic disturbances find the Balance Model, which appears in the second stage, to be of particular assistance, as the visualization helps clarify issues and prepare patients for self-help. These patients are often astonished when they put the events that have happened in their lives in the past few years into the four areas of the Balance Model, see them as catalysts, and understand the influence they have had in changing their lives.

Stage 3: Situational Encouragement

This stage emphasizes the development of specific resources. The most important aspect of the therapeutic relationship is the reflection of strengths. This aspect keeps the focus on the available capacities of the patient and his or her reference person in order to build a new relationship with the conflict partner. In any case, the patient and therapist look together at those capacities that correspond to this relationship with the conflict partner, and work through the meaning of these capacities. Rather than criticism of the partner, situational encouragement and appreciation are used based on what has been learned during the first and second stages. The patient and therapist organize the resources previously established in the Balance Model as well as areas that have been touched upon only briefly as well as potentialities and desires which have not yet been fulfilled. In this context, the relationship to the reference person is particularly valuable, whether it is discussed actively or only indirectly, especially in working toward the use of self-help. Psychoeducational information about the specific disorder and ways to address it, such as the use of medications, relaxation methods, and counseling services, are typical supportive tools during this third stage of the process. Nossrat Peseschkian

emphasized that in the third stage, the conflict partner was being prepared to withstand the criticism that he or she would face during the following stage of verbalization.

Stage 4: Verbalization

In order to move on past the speechlessness or the outpouring of speech so common with a conflict, the newly established communication must be carried over step by step into the social environment. In the fourth stage, the therapist and patient discuss both the positive and the negative characteristics of the experiences after which a relationship of trust was constructed during the third stage, which makes open communication possible. This stage is also when feelings and actual capacities are revisited and early experiences are remembered, organized according to themes, and made conscious. Together, the therapist and patient investigate the transference of wishes, expectations, and fears that were experienced with persons earlier in life or feelings that the therapist recognizes as signals indicating painful content. This process demands that the therapist be open and ready to be a confrontation partner and that he or she show a respectful attitude when the patient experiments with changes in behavior within the therapeutic relationship at this early stage. The therapist supports the patient in achieving a balance between politeness and openness and in taking the responsibility for change. The focus on the themes of the central conflict, the work on the key conflict, the politeness-honesty tradeoff, and the active involvement of the reference persons in the therapy through the patient are the tasks that belong to the fourth stage. The therapist presents the concept of the family group and, if appropriate, brings the family into the therapy. The family concept and unconscious basic conflicts are worked through at this time. Verbalization means that the time has come for an open discussion after which each recognizes the strengths of the other through observation of the situation, analysis of the contents of the conflict, and mutual encouragement. The conflict partner is now in a position to accept criticism or, at the least, to be able to speak about it. Experience shows that many people are inclined to speak about a problem immediately and to hurt the other person by doing so, after which the other person must be strengthened through many hours of encouragement.

Stage 5: Goal Expansion

The patient is advised to consider the following question: "What will I do when I no longer have this problem?" This stage accompanies the patient from the beginning. This goal setting helps prevent relapse, leads to development that is more proactive than reactive, and prevents the patient from resuming the use of symptoms as a means of relief after the completion of successful therapy. The patient is therefore guided to detach him- or herself from the therapist and to develop new capacities that he or she had neglected in the past. The patient will develop micro- and macroscale goals together with the therapist. Goals for the near future can be determined using the Balance Model.

Fields of Application of Positive Psychotherapy

To understand both the individual fields of application of PPT and its breadth, we must begin with the assertion and vision of Nossrat Peseschkian about its goals. He saw it as a special challenge to be able to treat people from all classes and cultures psychotherapeutically and therefore to recognize and make use of the uniqueness and the capacities of each person. This aim required the selection of concepts that would be useful for short-term therapy and understandable to everyone. Self-help would be placed in the foreground with every therapeutic step, and the connections that lay behind the complaints would be transparent to the persons involved.

> In other words, the patient is not only the sufferer of his illness but also is employed as a therapist himself…The essential difficulty of many patients is less a question of inadequate motivation to seek out a psychotherapist than of uncertainty about which psychotherapist is competent to deal with which kind of disturbance. This question can only be answered on the basis of a more comprehensive

> system which can bring together the multitude of existing psychotherapeutic orientations and assign them weights in accordance with their strong points. We present such a system in Positive Psychotherapy, which is not only a psychotherapeutic method but also a metatheory... It is not the patient who must adapt to a methodology he happens to be presented with, but vice versa: the methodology is selected in accordance with the changing psychotherapeutic needs of the patient. This flexibility permits the handling of all psychological and, in a broader sense, psychosomatic illnesses and disturbances. [3], p. 4f

In keeping with the emphasis on transcultural understanding, this form of psychotherapy is notable for combining traditional Eastern applications with current psychotherapeutic methods of the "Western" world. Peseschkian combines with a theory and therapy both systematic and structured in the Western sense the employment of proverbs and stories from different cultures, which can be used for association, support, or confrontation to broaden the patients' viewpoints about their own stories, to relativize their conflict situations, and to facilitate their psychodynamic understanding.

> One of my concerns in writing this book was to unite the wisdom and intuitive thinking of the East with the new psychotherapeutic knowledge of the West. Not only the contributions of the great religions which are of psychotherapeutic importance, but also the wisdom of Oriental and Western philosophers and scientists are considered in the light of Positive Psychotherapy. Our intention in doing so is to address not only the intellect but also the capacities for intuition and fantasy, for emotion and sense perception, and the ability to learn from the experiences of tradition. [3, p. 5]

Along with the main indication for PPT as a method of therapeutic and clinical treatment, it is also applicable in counseling, education, and prevention (the educational-preventive application of Positive Psychotherapy). In this context, the particular strong points associated with this method are (a) working with death and bereavement, (b) psychodynamic stress therapy based on PPT, (c) working with burdensome life events and psychological traumas, (d) preparation for partnership and marriage, (e) positive psychodrama (PPD), and (f) its application in general medicine and psychosomatic medicine. These uses give PPT a broad applicability for doctors, psychologists, and pedagogues.

By 1979 [39], four main fields of application had been formulated: psychotherapy, education, self-help, and transcultural problems. Based on these fields, the applications of PPT can be subdivided into three main categories:

Psychotherapy and Treatment

PPT has been proven effective and is applied for the treatment of following disorders (according to ICD-10): (F10-F19) Substance use disorders, (F30-F39) Mood [affective] disorders, (F40-F48) Neurotic, stress-related and somatoform disorders, (F50-F52) Behavioral syndromes related to physiological disorders and physical factors, and (F60-F69) Personality and behavioral disorders [40–42]. PPT has been employed successfully side by side with classical individual therapy in the settings of couple, family, and group therapy. There is experience with the application of PPT to psychiatric disorders [11]. Specifically, the use of stories and anecdotes has proven very effective in working with (post)psychotic patients and in group settings in psychiatric hospitals.

Self-Help

The greater part of Nossrat Peseschkian's books is intended to be used by laypersons for self-help. Among those books are "The Psychotherapy of Everyday Life" (1977 German, 1986 English), about dealing with misunderstandings; "In Search of Meaning" (1983 German and 1985 English), about coping with life crises; and "If You Want Something You Never Had, Then Do Something You Never Did" (2013), about interpersonal conflicts, as well as several books not yet translated from the German. Through courses for counselors, professional counselors can study and be certified to apply Positive Psychotherapy in conflict moderation and to stimulate self-help in conflict situations.

Outside of Psychotherapy

PPT has been applied in numerous and varied situations: in education and schools [43]; in management training [44]; in coaching; in various counseling settings; in seminars for preparation for partnership or marriage; in recruiting; in trainings for teachers [43], officers and politicians[45]; in mediation for jurists and mediators; in intercultural trainings; in time management [46]; in naturopathy and order therapy [also known as mind-body medicine] [47]; in stress management and coping [48]; in burnout prevention [49]; in the armed forces and society [45]; in the psychology of religion [27, 50, 51]; and in supervision [52]. Other areas include coaching, family counseling, and general counseling.

Development of Positive Psychotherapy and Its Current Organizational Structure

The *history of the development* of Positive Psychotherapy can be divided into five periods that roughly correspond to decades.

In retrospect, one can identify *the 1970s* as the foundation years in the development of the method. The fundamentals of PPT took shape during these years. These fundamentals were used with many patients and their families at this time and were tested and presented at international lectures both inside and outside Germany. Four of the five fundamental books of PPT were published during this period: "Psychotherapy of Everyday Life" (originally "Schatten auf der Sonnenuhr," 1974); "Positive Psychotherapy" ([original German] 1977); "Oriental Stories" ([original German] 1979), about the application of Eastern stories in Positive Psychotherapy, and "Positive Family Therapy" ([original German 1980).

The same period saw the first postgraduate trainings in PPT with the founding of a training institute in 1974 (the forerunner of the Wiesbaden Academy of Psychotherapy), its recognition by the Medical Chamber of Hesse for the psychotherapy residency training in 1979, and the founding of the German Association for Positive Psychotherapy in 1977—the first national PPT association in the world.

During the 1980s, more books were published, such as "In Search of Meaning" (German 1983, English translation 1985b). The method was systematized further in collaboration with some young colleagues. Hamid Peseschkian's dissertation, presented in 1987, was the first dissertation dealing with PPT. The first interview in PPT was first structured in this dissertation; a questionnaire for this first interview was presented, and a psychodynamic study of it was undertaken. This first-interview questionnaire was published shortly thereafter in 1988 (with minor modifications) together with the questionnaire in PPT, the WIPPF [53]. This precursor of the later semi-structured psychodynamic first interview was one of the first in psychodynamic psychotherapy. The last of the basic works of Nossrat Peseschkian, "Psychosomatics and Positive Psychotherapy" [7] (1991 German and 2013 English translation), also belongs to this period. The book presents a structured, psychodynamic model of illness and the treatment of different psychic and somatic disorders.

Nossrat Peseschkian began his travels for seminars and continuing education in PPT during the 1980s, predominantly in the developing countries of Asia and Latin America. This period coincided with the translation of the most important works of PPT into English. In addition, Nossrat Peseschkian undertook seminars in management training and in coaching, which provoked a great interest in the comprehensibility and applicability of PPT to these contemporary fields.

The international spread of PPT during the 1980s received a great impetus due to the radical political changes in Central and Eastern Europe *during the 1990s*. PPT attracted great interest in these cultures, which lie not only geographically but also psychologically between East and West. The organized working methods and the thirst for knowledge of Eastern European colleagues led to the systematization of seminars beyond Germany. More than 30 centers, beginning in 1990 in Kazan, Russia, and the first national associations for Positive Psychotherapy were established out-

side of Germany in Bulgaria (1993), Romania, and Russia.[6] PPT was further internationalized through the founding and registration of the International Center for Positive Psychotherapy in 1996 as a non-profit organization, the forerunner of today's World Association for Positive and Transcultural Psychotherapy (WAPP). These events occurred simultaneously with the founding of the European Association of Psychotherapy (EAP) in Vienna in 1990, which established the independent profession of psychotherapy and the appropriate legal framework. Representatives of PPT have been involved in this umbrella association from its beginning.

In the German-speaking countries, a lively discussion was sparked at least in part by a publication by Klaus Grawe [18] and the debate regarding the law for psychotherapy, a discussion about the broad questions of the effectiveness of the various methods of psychotherapy. Nossrat Peseschkian followed the spirit of the time and undertook an exhaustive "Effectiveness Study of Positive Psychotherapy" with his colleagues, which was distinguished by receiving the Richard Merten Prize [40]. This study was able to demonstrate the practical effectiveness of PPT.

In the *2000s*, the influence of the generations of PPT students who themselves took up teaching began to be evidenced. In 1999, a first international training curriculum for advanced studies in PPT was published on the experiences in different countries before. In 2000, the first International Training for Trainer in Positive Psychotherapy started. This broader development was formalized so that in Germany the Wiesbaden Academy for Psychotherapy (WIAP) received governmental license for the postgraduate education of psychological psychotherapists and child and adolescent psychotherapists. The German Psychotherapy Law (from 1998) led to increased developments in the curriculum and systematization of both basic and advanced training in PPT, which was also reflected beyond Germany. The WIAP Academy became one of the leading postgraduate institutes in Germany. Seminars at the basic level have been taking place in Eastern Europe for several years, and new concepts have been developed. In particular, PPT has moved into areas outside medicine, primarily school and university education as well as management training and coaching. National PPT associations in Germany, Russia, Bulgaria, Romania, the Baltics, Kosovo, Ukraine, Turkey, and Ethiopia developed further. PPT was also active through additional local or regional training centers in Albania, Armenia, Austria, Northern Cyprus, the Czech Republic, Poland, the Netherlands, Latvia, North Macedonia, Russia, China, and the UK. Seminars and lectures on PPT have taken place in more than 80 countries to date. The first university graduate program (Master's degree) in PPT was completed in 2005 at UTEPSA University in Santa Cruz, Bolivia. PPT is also included in curricula for psychology and psychotherapy at universities in Bulgaria, Russia, Ukraine, and Turkey.

WAPP has established international standards and guidelines for a four-stage education: basic and master training, and basic and master trainer (see http://www.positum.org/). Since 2000, annual International Training Seminars and since 1997 seven World Congresses have taken place. The Peseschkian Foundation (International Academy of Positive and Transcultural Psychotherapy [IAPP]), founded in 2005 by Manije and Nossrat Peseschkian, promotes international activities and administers the International Archives of Positive Psychotherapy.

Since 2010, PPT has entered the next stage of its development which is continuing until today (2025). WAPP, as the international umbrella organization of PPT, is registered as a non-profit and non-governmental organization under German law. PPT is an officially accepted modality of the European Association for Psychotherapy (EAP). The European Federation of Centers for Positive Psychotherapy (EFCPP) is a European Wide Organization (EWO), a European Wide Accrediting Organization (EWAO), and, through the IAPP, a European Accredited Psychotherapy

[6]The two authors of this chapter supported this development with extended stays in Eastern Europe: Hamid Peseschkian in Russia from 1991 to 1999 and Arno Remmers in Bulgaria from 1992 to 1995, and in Romania during 1996.

Training Institute (EAPTI) of the EAP. Trainees can receive the European Certificate of Psychotherapy (ECP) in the modality of Positive Psychotherapy. At present (2025), WAPP has more than 2,900 members in 64 countries. Training (psychotherapy postgraduate education) is taking place in 22 countries on three continents. Since the outbreak of recent wars and military conflicts, WAPP has established several support projects for psychotherapists affected by war and working with traumatized people. The political and societal challenges of the last decade have encouraged positive psychotherapists and positive consultants to engage more actively in society as "bridge-builders" as it was originally envisaged by Nossrat Peseschkian [54].

Research, Dissertations, and Publications

The first publications in the area of PPT date back to 1974. Since then, this method has been presented in numerous books, scientific works, and other publications.

Research

PPT fulfills the four principles postulated by Grawe [18] for the effectiveness of psychotherapy: (i) activation of resources, (ii) actualization, and (iii) management of problems, and (iv) therapeutic clarification. According to Grawe's model, PPT is a classical, integrative form of therapy [6, p. 9]. Substantiating this idea was the purpose of a wide-ranging effectiveness study that was carried out between 1994 and 1997 under controlled conditions. Thirty-two members of the German Association for Positive Psychotherapy undertook this study of effectiveness and quality assurance of PPT under the leadership of Nossrat Peseschkian, Karin Tritt, and Birgit Werner. This study was the first of its kind [40].

This longitudinal effectiveness study investigated the effectiveness of PPT under the conditions of daily clinical practice and compared recipients to a control group of patients with somatic illnesses who were on a waiting list for therapy. The recipients were treated by 22 therapists trained in PPT (15 physicians, 3 degree-holding psychologists, and 4 degree-holding teachers) with a median age of 45 and median professional experience of 7.7 years, all of whom were members of the German Association for Positive Psychotherapy. For a determined period of time, all patients taken into therapy were added to the sample. A total of 402 male and female patients were included: 23.6% with depressive disorders, 19.8% with anxiety and panic disorders, 21.2% with somatoform disorders, 20.5% with adjustment disorders, 8.2% with personality disorders, 3.4% with addictions, and 3.4% newly diagnosed with somatic disorders. In the design of this longitudinal study, 110 patients and a comparable control group of 771 other persons were questioned at the beginning and end of the study. A second part of the study contained a retrospective interrogation of the patients after the termination of PPT at intervals of 3 months to 5 years in three groups of 84, 91, and 46 patients. The choice of the battery of psychometric tests (SCL-90 R, VEV, Gießen-Test, WIPPF, IPC, IIP-D, GAS, BIKEB) was arrived at by collaborating with the participating university in consultation with Klaus Grawe and other researchers.

The patients who were treated with PPT showed a clear reduction in symptoms and an improvement in their emotions and behavior. Significant improvements ($p \leq 0.005$) in the symptoms of patients treated with PPT can be demonstrated by comparing the results of the Symptom Checklist (SCL)-90 before and after the treatment. A control group drawn from patients on the waiting list showed no significant change in their symptoms ($p \geq 0.05$). The calculated mean effect size of the study, $e = 0.476$, is a good indication of the positive effects of the treatment when performed under controlled conditions as presented in this study. The stability of the effect of the therapy can be seen through the comparison between the catamnestic and post-study measurements, which indicate no significant difference ($p \geq 0.05$; VEV: $F = 1179$; SCL-90-R: $F = 2473$) [40].

The discussion touched on the dilemma between an experimental design with high internal validity and a study pursued under controlled conditions with high external validity. The researchers suggested that in consideration of the unfortunate lack of such effectiveness studies, the experimental design devised for this study can be considered one of its strong points [40].

This computer-aided quality assurance study of PPT was awarded the Richard Merten Prize in 1997. The Richard Merten Prize is one of the best-endowed prizes in the health care sector in Europe and has been awarded by its trustees since 1992. The foundation's intention is to recognize outstanding work that leads to the improvement of medical/pharmaceutical/nursing treatment and that represents an important contribution to medical, social, sociopolitical, or economic progress in the health care sector.

Academic Works (Dissertations, Theses)

The basis for the multifaceted applicability and the cultural compatibility of PPT consists of the many colleagues who are stimulated and encouraged to undertake scientific work. Additionally, after coming into contact with PPT, many practitioners, no longer encumbered by the limitations and requirements of school, find their interest in publishing revived. According to available information, some 5 postdoctoral dissertations and approximately 25 doctoral (Ph.D.) dissertations have appeared on PPT, mainly from Germany, Russia, Bulgaria, and Ukraine. Some 40-50 Bachelor's and Master's theses have also been written on the subject.

The major focus has been on the psychosomatic, medical, psychiatric, psychological and pedagogical aspects of PPT and applications to these fields, which can provide a foretaste of future scientific work. A survey of the themes of these academic works shows the multifaceted clinical and nonclinical areas of applicability of PPT and the emphasis on certain models. Alongside works dealing with psychosomatics in various organ systems, we find comparative transcultural research. Distinctive features of the therapeutic relationship and its portability into educational contexts had been focused. Some of these scientific studies are based on the social-pedagogic context, thus bringing out the applications and possibilities of "Positive Pedagogy."

Publications

Publications on PPT consist of the wide-ranging source material written by its founder, Nossrat Peseschkian, and of the work of his students. These scholarly publications have been joined by works of popular science that have appeared in diverse periodicals and do not appear in lists of scientific literature.

Nossrat Peseschkian, as the founder of Positive Psychotherapy, wrote 29 books in German that have been translated into as many as 23 languages. The most widespread book is "Oriental Stories as Tools in Positive Psychotherapy: The Merchant and the Parrot." Other core books are "Psychotherapy of Everyday Life," "Positive Psychotherapy: Theory and Practice," "Positive Family Therapy," and "Positive Psychotherapy in Psychosomatic Medicine." In his final years, Nossrat Peseschkian published a number of self-help books dedicated to various areas of life.

Starting with the founding of the German Journal of Positive Psychotherapy in 1977, colleagues in PPT have been encouraged to publish the results of their research and share their cases. Additionally, the source publications of Nossrat Peseschkian began to accrue secondary publications beginning in the 1990s. As new national associations for Positive Psychotherapy have been formed in various countries during the last 20 years, journals of PPT have been founded in Russia, the Ukraine, Bulgaria, and Romania. In the past ten years, an increasing number of articles and books have been published by the "second" and "third" generation of positive psychotherapists (e.g., I. Kirillov, M. Goncharev, R. Cieselski). Since 2021, a peer-reviewed online journal "The Global Psychotherapist" has been published biannually.

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Positive Psychology: An Introduction

3

Tuğba Sarı

Introduction

> Psychology isn't just about medical science or curing negative things. It is about education, work, marriage and even sports. What I want to see is that psychologists try to help people empower themselves in all areas. [32]

When the history of psychology is examined, it is seen that the first personality development theories approach the human from a pessimistic perspective or treat the individual as a passive organism shaped by external influences.

The Positive Psychology movement gained momentum with Martin Seligman's speech when he was elected as the president of the American Psychological Association in 1998. In this speech, Seligman emphasized the need to reshape the trajectory of pathological-oriented psychology [32]. In the following periods, the Board of Positive Psychology was established under the chairmanship of Seligman, followed by the establishment of the Positive Psychology Network and the first Positive Psychology Summit was held in Washington DC. Subsequently, Seligman established the Center for Positive Psychology at Pennsylvania State University. In 2005, the first positive psychology graduate program was opened at Pennsylvania State University. The Journal of Positive Psychology, the first positive psychology journal, started its publication in 2006 [11].

According to Seligman and Csikszentmihalyi [34], two important pioneers of positive psychology, the field of psychology was mainly focused on abnormal behaviors and their efforts to reduce them. Especially after the Second World War, psychology workers have tried to understand and eliminate behaviors arising from mental disorders as health professionals. With the emergence of humanistic psychology and the introduction of Maslow and Rogers' views of human nature in the 1950s, the psychopathology-oriented approach was replaced by a focus on positive human characteristics. As a matter of fact, by the end of the 1990s, a positive psychology approach developed as a new psychology movement that examines the strengths of human beings. There are several definitions of positive psychology by different scientists. For instance, according to Seligman and Csikszentmihalyi [34], positive psychology is a scientific discipline that examines human functioning and development in a multidimensional way in a biological, personal, relational, cultural, and global context. In this context, psychology is not considered just a science of pathology, weakness, and damage; it is also about strengths and virtues. Similarly, according to Sheldon and King [37], positive psychology is nothing more than examining the

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_3

strengths and virtues of an ordinary person. Peterson [24] mentioned that positive psychology is a scientific approach to studying human thoughts, feelings, and behavior, with a focus on strengths instead of weaknesses, building the good in life instead of repairing the bad. Gable and Haidt [14] stated that positive psychology is an area that contributes to the study of the conditions and processes necessary for the maximum functioning and development of not only people, and groups and institutions as well. As can be understood from the definitions mentioned above, positive psychology deals with the strong characteristics, virtues, and potentials of individuals.

In this context, the most basic assumptions of positive psychology regarding human nature can be listed as follows: (1) human is an active entity capable of directing his/her own development, (2) human has the intrinsic power necessary to regulate and change human behavior, (3) the individual develops behavior in the desired direction and at the same time activates the motivation to connect to life [1]. Acting on these assumptions about human nature, the positive psychology movement examines how and under which conditions positive characters, positive emotions are formed, and tries to present the findings in scientific ways. On the other hand, it does not deny the negative characteristics of the individual. However, it emphasizes that it is as important to build life on the traits that make the individual strong as it is to repair the bad thing.

Positive psychology does not reject people's problems, does not ignore the stress of people, and does not oppose people's attempts to understand what good life means. On the contrary, it aims to complement and not replace ordinary psychology by expanding studies on human growth and development [22]. In contrast to traditional psychology, positive psychology has been primarily concerned with the strengths and positive personal characteristics of human nature and has sought to draw attention to improving the quality of life [34]. Positive psychology brings together research findings that provide good living [23].

Health Model

The main purpose and functions of the traditional approach using the disease model have been to solve problems, eliminate negative effects, and treatment [15]. The health model focuses on health protection and development. In understanding human personality, positive psychology uses the health model in contrast to the disease model used by traditional psychology. The perspective of positive psychology on human nature coincides with the World Health Organization (WHO)'s definition of health. According to WHO, health is defined not only as the absence of disease and weakness, but also as a state of complete physical, mental, emotional, social, and spiritual well-being. Similarly, mental health is defined as being aware of one's own abilities, overcoming the stress that occurs in life, being productive and useful in business life, and contributing to society in accordance with their abilities [38]. In positive psychology, the aim is to reveal the characteristics that improve the well-being and joys of individuals by searching for things that make life valuable and worth living [20]. In this context, positive psychology focuses on the adoption of the health model by moving away from the disease model. The following figure presents the proposed health model in the context of positive psychology (Fig. 3.1).

When the literature on psychology is examined, it is seen that besides the theories explaining the pathological aspects of human behavior, models related to positive characteristics such as hope, optimism, well-being, and happiness are limited, and the development of these models is neglected. In this context, it has been tried to develop various models related to the positive experiences and characteristics mentioned in positive psychology studies and to understand the structure of well-being in a nutshell.

From this point of view, there are many positive characteristics that are discussed within the framework of a positive psychology approach. Psychological resilience, subjective well-being, self-efficacy, psychological well-being, hope,

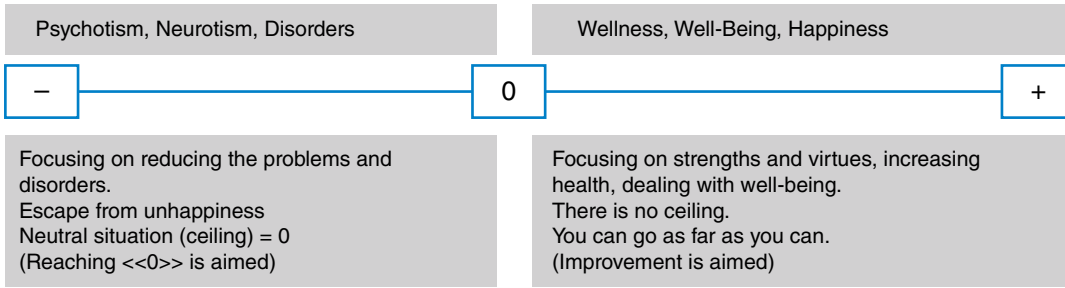


Fig. 3.1 Health model in the context of positive psychology

commitment to school, job satisfaction, relationship satisfaction, character forces, self-esteem, optimism, gratitude, post-traumatic growth, meaning of life, self-compassion, conscious awareness as examples of these characteristics shown. Positive psychology deals with many concepts that affect individual happiness and psychological well-being. In this section, subjective well-being, life satisfaction, psychological well-being, and optimism, which are the most researched concepts in positive psychology, are discussed as the basic building blocks. First, the concept of well-being, which tries to explain the various aspects of human happiness, is emphasized.

Well-Being

What is well-being? Being happy in life is an important expectation of people. In the field of positive psychology, people's happiness is examined with the concept of well-being. Well-being is conceptualized as self-realization and fully functioning from an *eudemonic* perspective. On the other hand, hedonic perspective is considered as self-assessment of the individual's own satisfaction and happiness [9]. The concept of well-being is considered in two basic dimensions: psychological well-being and subjective well-being. The concept of psychological well-being is represented by the concept of *eudemonic*, and subjective well-being is represented by the concept of *hedonic* [26]. Subjective well-being is not considered as the equivalent of psychological well-being and being healthy. Some people with mental health problems may say that they are happy. Looking at these statements, it cannot be concluded that such people are healthy [8]. Therefore, it should not be forgotten that subjective well-being is the only one aspect of psychological health.

Subjective Well-Being

According to Diener [4], who made preliminary studies about subjective well-being, subjective well-being is the both cognitive and affective self-evaluations of the individual for their lives. The situation reached as a result of the assessment of the individual gives information about the well-being of the individual. Since the cognitive and affective evaluations of the individual are based on self, this concept is described as subjective well-being. There is life satisfaction in the cognitive dimension of subjective well-being and positive and negative affect in affective dimension [3]. *Life satisfaction* is the evaluation of an individual's own life in general about whether it is good or bad [21]. According to Diener et al. [7], life satisfaction includes satisfaction from life, satisfaction from the past, satisfaction from the future, and the views of one's relatives about that person's life.

Positive affect is the feeling of positive emotions such as eager, energetic, cheerful, calm, peaceful, lively, and grateful. *Negative affect* includes the feeling of bored, sad, anxious, worthless, angry, and guilty. Intensive positive affect includes the individual's positive attitude toward other people and life activities and positive mood. Intense negative affect causes the per-

son to perceive his/her life as bad [5]. From this point of view, when individuals feel more positive emotions and less negative emotions, it may mean that they have high subjective well-being [4]. In fact, negative emotions such as anger, hate, and fear have evolutionary advantages and functions. They are developed to take the necessary measures to survive in life-threatening situations. The problematic situations are the frequent and intense experiences of these emotions, and the positive emotions are less and unnoticed.

As a matter of fact, laboratory studies conducted by Fredrickson [13] reveal that positive emotions extend individuals' thoughts and actions repertoire, reduce the effects of negative emotions, and improve psychological soundness. Positive emotions help the individual to recognize and develop internal resources. The constructive effect of experiencing positive emotions is shown in Fig. 3.2.

From this point of view, individuals with high subjective well-being are generally expected to have more positive emotions, and experience less negative emotions such as sadness, anxiety, and anger. Although subjective well-being plays an important role in assessing a person's well-being, there is a point to note: Subjective well-being alone is not sufficient to define mental health. In other words, subjective well-being is not synonymous with psychological health [8].

Although not universally considered, there are some factors that affect subjective well-being. According to Diener [3], these variables are divided into three groups as demographic, personality, and social variables. There are many studies investigating the relationship between demographic variables such as age, gender, educational status, income status, and subjective well-being. Most of these studies show that these characteristics explain only a small part (10%) of subjective well-being [21]. In terms of personality traits, many researchers argue that personality is one of the most powerful influential factors of subjective well-being. Self-esteem, optimism, and frequently felt positive emotions in personality traits were found to be associated with life satisfaction and therefore subjective well-being [8]. It is stated that personality traits have a 50% effect on subjective well-being [21]. In the context of the culture considered as social variables, it is stated that individuals with individualistic cultures have higher levels of subjective well-being in individualist cultures because they can pass their interests and wishes to their lives more [8]. Research in the context of being associated with other social variables, other people, indicates that happy people have more social relationships; however, unhappy people have less social relationships [6, 19].

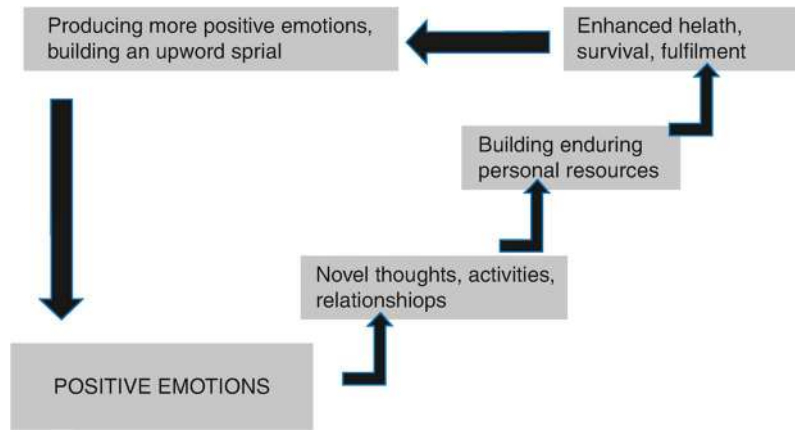


Fig. 3.2 Table showing the widening effect of positive emotions. (Adapted from Fredrickson's [13] Broaden-and-Build Theory of Positive Emotions)

Psychological Well-Being

As mentioned earlier, subjective well-being is based on hedonism and psychological well-being is based on functionality. According to Ryff [27], which provides a comprehensive perspective on psychological well-being, psychological well-being cannot be explained by not only having mental disorders and not having a certain problem. Psychological well-being requires the ability to perform certain life functions.

Accordingly, for an individual to be psychologically good, he/she must have self-esteem, good relationships with others, discovering the meaning of life and living in this direction, autonomy, and ongoing growth/development. Starting from this, psychological well-being model consists of six structures. These are: (1) Self-acceptance (having a positive attitude toward oneself and past life), (2) Positive relationships with others (establishing warm and reliable relationships with others), (3) Autonomy (feeling of making your own decisions, freedom, (4) Environmental mastery (capacity to create a suitable environment according to the individual's personal and mental conditions), (5) Individual development (having the sense that the individual develops and grows as an individual), and (6) The purpose of life (having a sense of finding meaningful life) [28]. According to this model, psychological well-being is related to better performing of human functions and therefore to individual development and self-realization [29].

Many factors are effective in achieving psychological well-being in individuals. According to a study, psychological well-being increases with extroversion, education, and honesty [17]. In another study, psychological well-being was found to be negatively associated with depression, anxiety, and anger [30]. It has also been shown that psychological well-being has a protective effect against negative conditions such as depression [16] and neuroticism [17]. In parallel, it is seen that individuals who use negative coping strategies less have a higher level of psychological well-being [18]. As a result, studies show that psychological well-being is associated with many positive conditions.

Seligman [33] introduced the concept of *authentic happiness* which has three basic elements: (1) High level of positive emotions and life satisfaction, (2) Quality of life including continuous assimilation and flow; providing meaningful life, and 3) A meaningful life that allows the individual to serve something greater and stronger than himself. The first dimension is explained by hedonistic approach (subjective well-being), and the second and third dimensions are included in the functionality approach (psychological well-being). While engaging in activities in the first dimension produces positive emotions in the individual, engaging in activities in the second and third dimensions adds meaning and value to an individual's life. From this point of view, it is possible to say that individuals' experiences that increase both their subjective well-being and psychological well-being increase their happiness and prepare the basis for living in totality and balance.

Seligman [39] devised his concept of well-being using the PERMA model, which is a framework that he developed. This model evaluates human happiness and well-being using a total of five dimensions [40].

1. P—Positive Emotions: Positive emotions are one of the key factors in determining happiness. When individuals experience feelings of joy and gratitude frequently, it can significantly enhance their overall well-being [39].
2. E—Engagement: The concept of being fully devoted to an activity or task and losing track of time can be described as a state of being fully immersed and engaged in it. By harnessing one's strengths, an individual can concentrate and participate in the activity with great enthusiasm. This dedication leads to a sense of deep fulfillment and contentment [40].
3. R—(Positive) Relationships: As individuals, it is crucial to foster positive and nurturing connections with others to enhance our sense of worthiness and find purpose in life. Developing robust and supportive social networks can provide a sense of belonging and contribute to overall well-being [41].

4. M—Meaning: Understanding that life has a purpose enables individuals to feel that they are part of something greater than themselves. By creating their own meaning and purpose, individuals are better equipped to handle life's challenges [42].
5. A—Accomplishment: Setting and attaining personal objectives has a significant impact on an individual's well-being. Accomplishing goals and milestones boosts an individual's self-assurance, which ultimately supports their overall well-being [43].

Seligman identifies these five elements as essential for achieving happiness and a satisfying existence. The PERMA model provides a roadmap for increasing an individual's well-being.

Learned Optimism

In Seligman [35], foundations of the concept of "learned optimism" are based on the concept of learned helplessness. Therefore, in this section, the concept of learned helplessness will be included before the concept of learned optimism is explained.

Seligman developed the *learned helplessness* hypothesis during his experiments with dogs in the 1960s. In the experiments conducted by Seligman and Maier in 1967, it was observed that the dogs did not react to shock evasion when shocked. In these experiments, Seligman divided dogs into two groups: one group was given the opportunity to escape from the shock, while the other group showed no response regardless of the shock. At the end of these experiments, it was observed that the dogs in the second group were unresponsive to the shock given after a while and did not attempt to escape. After many experiments in the following years, Seligman explained this unresponsiveness in dogs with the concept of learned helplessness. According to this, learned helplessness means that the organism does not show and is passive in similar situations where it cannot establish a relationship between a behavior that occurs and the result of this behavior. Starting from here, Seligman and Gilliham [36] conducted various experiments with people who thought that learned helplessness could be generalized to human behavior. For example, in an experiment, individuals were given some riddles to solve. Individuals were divided into two groups, as in experiments with dogs. A group was given riddles that were difficult to solve and sound waves disturbing the ear were given to the environment unless individuals could solve the riddle. These people have experienced that there is no way to avoid negative stimuli, just like in experiments with dogs. The other group of individuals were given questions that they could easily solve, and the annoying sound was withdrawn as they solved the riddles. Individuals in both groups were given new riddles again in the second part of the experiment after this part of the experiment ended. Although these riddles were easy, in the first part of the experiment, the individuals in the failed group did not attempt to solve the riddles.

Similar to the learned helplessness model, in the framework of the learned optimism model, optimism is handled by the individuals' way of explaining the events. It was found that individuals with an optimistic loading style had physical health, motivation, high morale, and low level of depression symptoms. Based on the findings of this research, he developed the idea that a transition from pessimism to optimism is possible. For the realization of this change, he proposes to use the Inverse-Thought-Result model developed by Alber Ellis and Beck's ideas [25].

The four elements that an individual should be aware of in the process of developing an optimistic way of thinking are evidence, alternatives, implications, and usefulness [31]. Evidence is to look for proof of distorted or unrealistic thoughts in negative explanations. Alternatives include screening for all negative causes. Implication is the discussion of the consequences of the negative/pessimistic explanation. On the other hand, the usefulness of the individual is to question whether the negative thinking about the reverse is destructive. If the individual realizes that the thought is destructive, he/she can easily oppose this thought or try to make it less destructive. For example, an individual who interprets the indif-

ference of one of his/her friends as being worthless can look for evidence for this idea, try to develop alternative thoughts instead of this idea, realize the results of this idea, and finally discuss how destructive this idea is. At the end of this study, instead of a pessimistic point of view, it can gain energy by reaching a more optimistic way of explanation.

As a result, Seligman argues that individuals can reduce their pessimism and increase their optimism by working on pessimistic thoughts that they have shown against unfavorable events. In this direction, he developed the concept of learned optimism and explained how to improve optimism. It is believed that the attribution of the individual to the events, in other words, the manner of explaining the events, is important. In a study conducted on the effectiveness of the optimism development program [2], it was concluded that the eight-session learned optimism development program was effective in increasing the level of optimism as a loading form.

Conclusion

Positive psychology is a discipline that focuses on protection and development of positive characteristics of individuals, positive experiences, and their mental health through programs that increase their subjective well-being [34]. Within the framework of the positive psychology approach, researches on the above-mentioned concepts of well-being continue to be conducted, and studies are carried out to understand and develop the positive aspects of human personality. In this context, according to Seligman and Csikszentmihalyi [34], the positive psychology approach conducts research in three important areas: (a) Working on what positive experiences of individuals are, (b) Working on what the positive characteristics of individuals are, (c) Working on positive institutions.

One of the criticisms of positive psychology is that positive psychology is more research-based in methodological terms, it gives more space to cross-sectional and correlational studies, whereas it gives less space to theoretical explanations. It is true that positive psychology focuses more on research and quantitative studies, but it should not be overlooked that there are studies of subjective well-being, such as adaptation theory, top-down and bottom-up theory, goal theory, and flow theory [12]. Positive psychology has also brought a different perspective to the science of psychology in general. Now, researchers do not only conduct studies on burnout in industrial psychology as well as work satisfaction; in addition to divorce and marriage problems in marriage, they also investigate marriage satisfaction. In this respect, positive psychology could be considered to have led to a paradigm shift in the world of psychology.

Another criticism is related to whether positive psychology is a new approach or not. As noted above, there are researchers and theories in the history of psychology that deal with the positive characteristics of man. Positive psychology seems very similar to the humanistic psychology put forward by Maslow (1971) in the 1970s, but it differs in various ways. Humanistic psychology is a qualitative method based on a phenomenological approach, examining humans and their behavior. Positive psychology, on the other hand, can be used in experimental and quantitative methods just like natural sciences [10].

Questions such as "What is good life?," "what is happiness?," and "what is well-being?" have been tried to be answered by different disciplines such as theology, philosophy, economics, and psychology. But even today, these issues are still an enigma. The positive psychology approach seems to be the youngest discipline seeking answers to these questions. To date, what has been put forward in this direction has shown a positive contribution of positive psychology to human well-being.

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Part II

Staying Positive Through Life

Positive Child and Adolescent Psychiatry

4

Consuelo Cagande, Alicia Barnes, and Salman Majeed

You're braver than you believe, and stronger than you seem, and smarter than you think. –Christopher Robin

Introduction

We learn from the Adverse Childhood Experiences Study (ACES) that persons who experience four or more adverse events in childhood have higher risks for several of the leading causes of death in adults. Researchers have studied significant factors for youth at risk for mental illness [1–4] as well as factors that can promote positivity in youth [5]. While there is a paucity of research on positive psychiatry in children and adolescents, there is a plethora of literature on wellness. Cultivating wellness and positivity among youth have been essential in promoting resilience in youth. Child and Adolescent Psychiatrists, Social Workers, and Psychologists have the skills in preventing and intervening earlier when this population has a lot more demands at home, school, work, and social arenas, including the media. Decades ago our focus was diagnosis and treatment but now we are preventing and screening which in this healthcare landscape is vital for resiliency. Resilience in children and adolescents can be greatly enhanced by paying attention to the value of cultural, sporting, and other activities, sensitive mentoring to build self-esteem and strengthen mental health, and open new social relationships.

Learning from Positive Psychology where it seeks to understand and promote well-being through assessment and interventions involving positive psychosocial characteristics (PPCs) in people who suffer from or are at high risk of developing mental or physical illnesses, it is very relevant in Child and Adolescent Psychiatry where prevention and early intervention can help the trajectory of a child's mental health and functioning. In their clinical handbook on Positive Psychiatry, Jeste and Palmer highlighted four main components:

1. Positive mental health outcomes (e.g., well-being)
2. Positive psychosocial characteristics that comprise psychological traits (resilience, optimism, personal mastery and coping self-

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_4

efficacy, social engagement, spirituality and religiosity, and wisdom-including compassion) and environmental factors (family dynamics, social support, and other environmental determinants of overall health)
3. Biology of positive psychiatry constructs
4. Positive psychiatry interventions including preventive ones. There are promising empirical data to suggest that positive traits may be improved through psychosocial and biological interventions [6]

Furthermore, Rettew [5] identified 12 domains of wellness in positive child psychiatry:

1. Positive attributes and traits
2. Nutrition
3. Physical activity and exercise
4. Involvement in structured activity
5. Music and the arts
6. Reading and limiting screen time
7. Parenting behavior
8. Parental mental health
9. Spirituality and religious involvement
10. Compassion and giving back to others
11. Mindfulness
12. Sleep

Applications

Positive psychiatry is increasingly gaining the attention it deserves. But is there evidence of significant outcome when applying positive exercises, especially in youth? How are they applied? There are many components of positive psychiatry that have been applied and studied.

A study ventured in replicating Seligman's Positive Psychology Exercises (PPEs) study and its long-term benefits. Mongrain et al. added a "positive placebo" which was the cognitive access of positive information about the self. They concluded that PPEs do lead to lasting increase in happiness in both control and placebo groups. However, changes in depression over time did not exceed the control condition. Activation of positive, self-relevant information rather than through other specific mechanisms seems to be an effective method with brief positive psychology interventions to enhance happiness [7].

Given negative self-evaluation is prominent in adolescents with depression, positive self-reflection may be a protective factor. One would expect positivity in this aspect would be low. Orchard et al. studied the specificity of positive and negative self-evaluation in adolescent depression. They concluded that positive self-endorsements were not impaired nor associated with the severity of depressive symptoms [8].

Life satisfaction (LS) is an attribute one tends to measure themselves at times. In adolescents, subjective life satisfaction can impact their academic performance. A study sought to find evidence for reciprocal association of life satisfaction and academic performance and how the relations may be shaped by positive and negative affective experiences in school. The results showed positive reciprocal causal relations between students' LS and grades. Therefore, LS is synergistic with better school grades and should have a role in positivity in the school curriculum [9]. This can further boost one's confidence and competence.

Rosenberg's study on "Promoting Resilience in Stress Management" (PRISM) on adolescents and young adults with cancer showed improvements in hope and benefit finding which are two adaptive coping skills that may mitigate long-term psychosocial risk. This was a randomized study comparing those in PRISM to usual care. PRISM taught stress-management, goal-setting, cognitive-framing, and meaning-making skills [10].

Interpersonal relationships can be precipitating factors for youth with depression and anxiety. Herres studied emotional reactivity in youth with anxiety disorders. Negative parent and teacher events can cause more negative affect reactivity. Negative peer events can cause less positive affect reactivity. Emotion regulation associated with negative events involving adults and addressing barriers to developing and maintaining positive peer relationships should be goals in therapy [11]. Self-control has shown to be effective in accomplishing goals. The science and practice of self-control study by Duckworth and Seligman

demonstrated that self-control "outdoes" talent in predicting academic success during adolescence [12]. Furthermore, family is the core unit that can either promote or demote positive thinking. The application of the constructive approach, not destructive, of "functional rehabilitation of the family" must be strongly emphasized in treatment [13].

Grebosz-Haring studied the effects of music, mainly singing versus listening, in hospitalized children and adolescents with mental illness. This pilot studied the efficacy of music-related interventions by measuring potential neuroendocrine (cortisol), immune (IgA), and psychological (mood state, health-related quality of life (HRQOL), well-being). Their finding was significant in demonstrating that singing led to a larger mean drop in cortisol and in HRQOL than those listening to music. Those listening, though, led to a higher mean positive change in calmness and improvement in well-being than singing [14]. Music therapy has been part of most inpatient settings but a larger study replicating this and having objective measures of the efficacy of music therapy would be ideal.

A cross-sectional study examined the predictive value of mindfulness and self-compassion for depressive symptoms, negative affect, and positive affect in adults. The Five Facets of Mindfulness Questionnaire (FFMQ) was used as a measure of mindfulness and the Self-Compassion Scale (SCS) as a measure of self-compassion. The study found a harsh attitude toward oneself had a great value in predicting the presence of psychological symptoms. Four of the five FFMQ facets (observe, describe, act with awareness, and non-reactivity) were significant predictors, suggesting that mindfulness is a more important predictor of positive affect than self-compassion, as measured by the FFMQ and SCS [15]. Mindfulness is not just for adults anymore. Applications of relaxation techniques such as mindfulness and yoga are now ubiquitous and effective. For example, relaxation therapy compared to watching relaxing videotape improved anxiety, including anxious behavior and fidgeting, in hospitalized children and adolescents with adjustment and depressive disorders. They also measured cortisol level and found it to decrease in the relaxation therapy group but no change in the video group [16].

Social media, internet, and screening time have received negative connotations, such as behavioral problems. Limiting screen time and more reading can improve language development, self-regulation, and academic achievements [5]. But applying innovative techniques using the internet can be effective for those who are not able to access mental health treatment timely. Internet-delivered cognitive behavior therapy (ICBT) has actually been around for approximately 20 years. Although a study by Andersson et al. found ICBT to be more effective than a waiting list and tends to be as effective as face-to-face CBT, it has not really been positively accepted by some patients who prefer using modern information technology as an adjunct to face-to-face therapy [17]. This is an area that earns itself for more innovative techniques and applications.

There is potential for the use of social media in promoting mental health in adolescents. O'Reilly's qualitative study surveyed adolescents, educational professionals, and mental health practitioners about their perception about the value of social media promoting positive mental health. As we mentioned, there are challenges with using social media and the internet for information about mental health but they can also be useful ways of providing education and positivity to further promote mental well-being according to O'Reilly's study [18]. Outcome studies have shown that positive psychology concepts are promising in pediatric oncology, promotion of sexual health and HIV/STD, pregnancy, and unprotected sex. Positive coping appraisals have been found to buffer the relationship between life stress and suicidality.

A psychiatrist can apply comprehensive assessments to include the strengths, weaknesses, interests, coping mechanisms, and interpersonal relationships of the child and adolescent. It is also imperative to apply it from a culturally-informed and trauma-informed lens. Furthermore, youth self-evaluations and life satisfaction assessment should be performed. Child

and Adolescent Psychiatrists in training (fellows), therapists, counselors, and settings such as schools and primary care should implement aspects of positivity into their routine evaluation, curriculum, and training.

Case Presentation

An 8-year-old boy, John (pseudonym), comes to the office with the presentation of ADHD. His primary caregiver is his father who holds two jobs, and lives with two other siblings. His mother lives with substance use disorder, is unhoused, and not involved in his care. He attends a public school that has implemented socio-emotional learning, that has mental health counselors in the school, and emphasizes the need for positive child-teacher interactions. His father is considered to have an authoritarian parenting style; however, his father creates space for open dialogue with his son. Additionally, the father makes every appointment for his son, and is seeking housing into a larger house that he can renovate with a city program that supports neighborhood development.

Carl Bell MD stated, "Risk factors are not Predictive Factors because of Protective factors." There are many aspects of this case we can learn from by applying Positive Psychiatry and by exploring positive childhood experiences that can mitigate the impact of adverse experiences such as:

1. *Comfort confiding in at least 1 parent about things that were bothering them*: Despite his father's authoritarian parenting style, John occasionally finds moments to confide in his father about his concerns, fostering a sense of trust and communication. (Positive psychosocial characteristics—psychological traits)
2. *Perception that at least 1 parent understood their problems*: John perceives that his father understands his challenges, evidenced by attentive listening and discussions about school and behavior. (Positive psychosocial characteristics—psychological traits)
3. *Rating of their relationship with their parents*: The relationship between John and his father is generally positive, marked by moments of affection, shared activities, and acknowledgment of John's achievements. (Positive psychosocial characteristics—psychological traits)
4. *Happiness at school*: John experiences happiness at school, facilitated by the supportive environment focused on socio-emotional learning and positive teacher interactions. (Positive psychosocial characteristics—environmental factors)
5. *Comfort with friends*: John feels comfortable and enjoys friendships at school, which contribute positively to his social development and sense of belonging. (Positive psychosocial characteristics—environmental factors)
6. *Perception of their neighbors' helpfulness*: Participating in a city program supporting neighborhood development has enabled John and his family to perceive their neighbors as helpful. This sense of community support enhances their overall well-being. (Positive psychosocial characteristics—environmental factors)
7. *Access to care*: The presence of mental health counselors in school offers him additional support. (Positive interventions and mental health outcomes, including preventive ones.)

In terms of ACEs, John's upbringing includes significant adverse childhood experiences, such as parental substance use disorder and homelessness. These ACEs increase his vulnerability to developmental and behavioral challenges, including ADHD. Understanding the impact of ACEs on John's life underscores the importance of supportive interventions and positive experiences to mitigate potential negative outcomes.

In managing John's ADHD, the holistic approach includes leveraging these positive childhood experiences. Emphasis is placed on maintaining and enhancing the supportive school environment, fostering positive parent-child relationships despite challenges, and integrating community resources for ongoing support.

Collaboration with mental health professionals to promote positive coping skills and school personnel is crucial for optimizing John's academic and emotional development.

Summary

John's case highlights the importance of recognizing and incorporating positive childhood experiences in mitigating the challenges associated with mental illness within a complex family environment, compounded by adverse childhood experiences. By leveraging supportive school initiatives, maintaining strong parent-child relationships, and cultivating community resources, John's overall well-being and developmental outcomes are optimized despite familial adversities.

Positive psychiatry allows the clinician to promote resiliency in youth. There are many components to positivity. Obtaining comprehensive information on the youth's self-regulation, life satisfaction, and self-evaluation in addition to the traditional clinical assessment can aid the psychiatrist in formulating a meaningful and individualized positive treatment plan (PTP). Additionally, the PTP should emphasize and target positive protective factors, positive interpersonal relationships, and emphasize family and community formation.

Key Points

- Positive mental health thinking through various means of application promotes resilience in youth.
- Positive psychology exercises (PPEs) in various forms (e.g., relaxation therapy, mindfulness, music therapy, stress management, self-evaluation, life satisfaction, innovative internet CBT) are applicable to youth.
- Including PPEs and positive childhood experiences in a patient's assessment and treatment plan is crucial to fostering resilience and improvement in youth depression and anxiety, and other behavioral problems.

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Positive Psychiatry in Midlife

5

Samidha Tripathi and Erick Messias

Introduction

The term "midlife crisis" was first used by psychoanalyst Elliot Jaques on a paper looking at prominent artists, among them Raphael and Dante, who experienced critical periods in their mid-30s and thus had what he called a "midlife crisis" [1]. Dante himself alludes to being in the middle of life in the majestic opening lines of the Divine Comedy:

> Midway upon the journey of our life.
> I found myself within a forest dark,
> For the straightforward pathway had been lost.

Indeed, the trajectory of psychological well-being when plotted over age has a U-shaped curve with its nadir at about the age 50 (Fig. 5.1) [2].

Similarly, the lifetime prevalence and risk of depression seem to peak between the ages 46 and 49 [3]. Taken together, these findings seem to confirm the notion of a midlife crisis occurring between the ages of 45 and 55. Interestingly, there is evidence for a similar "midlife crisis" among great apes [4] suggesting mechanisms beyond social and cultural pressures contributing to this phenomenon.

Midlife is also a pivotal age standing at the intersection of the decline in physical health, speed of processing, and working memory and the acquisition of knowledge, wisdom, and emotional regulation [5]. This convergence can foster a period of consolidation of life experience before the challenges of aging take hold.

The importance of understanding this key period in life led to the large epidemiological study Midlife in the United States (MIDUS) supported by the National Institute on Aging and available here: http://midus.wisc.edu/ [6].

The longitudinal findings on life satisfaction from the MIDUS study tell a somewhat different story than the U-bend. It shows that a majority of middle-aged adults are satisfied with their life and stay that way or even improve over a 10-year period. One possible explanation for the lower scores often reported in cross-sectional studies is that those in midlife have not yet met their goals and aspirations, and they see room for improvement and growth. In contrast, older adults may be closer to their peak in terms of goal attainment. Indeed, whereas present satisfaction is on the rise, and has not yet reached its peak in midlife, projected satisfaction about the future is on its

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_5

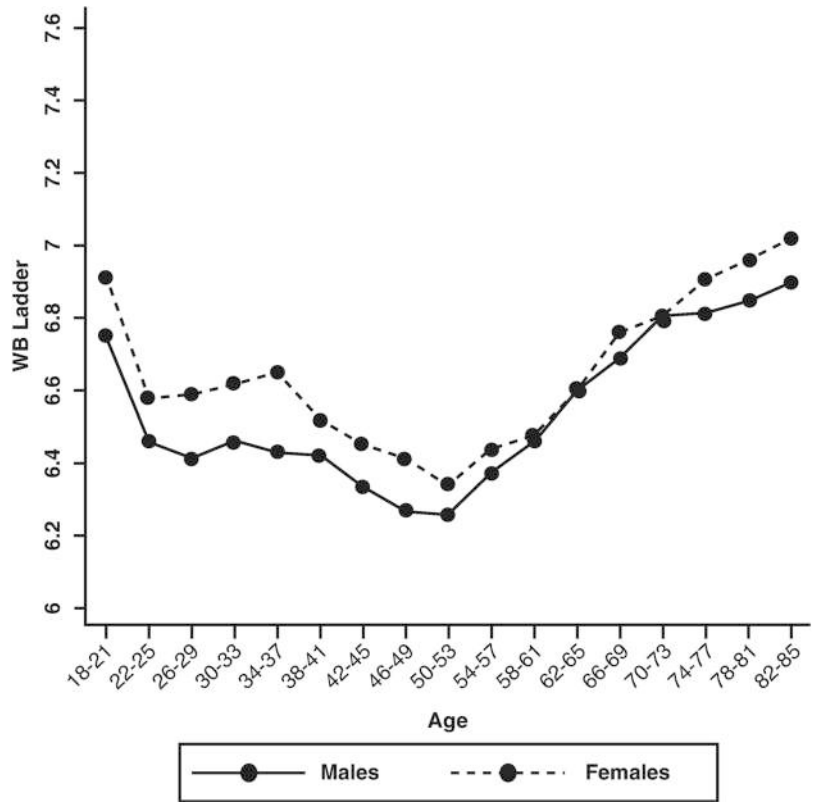


Fig. 5.1 Well-being through the lifespan [2]. Note the nadir around age 50 followed by a gradual increase from 55 to 85. (Figure reprinted with permission from the Proceedings of the National Academy of Sciences)

way down, and has not yet reached the nadir which does not occur until old age [5, 7].

Definitions

Midlife is usually defined as the central period of a person's life, commonly thought of as between the ages of 45 and 55. Some studies extend the concept to early midlife 45–55 and late midlife 55–65.

A *life event* is defined as an important happening in someone's life with an impact in several areas of their being. Midlife is a period with high density of life events, the most prevalent being:

1. Close family death.
2. Family health issues.
3. Major financial difficulties.
4. Major job changes.
5. And moving to a new place of residency [8].

Midlife crisis is described as an emotional crisis of identity and self-confidence most commonly observed in early midlife.

The notion that midlife crisis is a phase that most adults go through has been rejected. Predisposing factors include a history of psychological crisis and personality type [9].

Factor associated with resilience in midlife include [8] the following:

1. Growth and purpose.
2. Managing uncertainty.
3. Spirituality.
4. Emotional objectivity.
5. Adversity level.
6. Humor.
7. Changing philosophy.

Growth and Purpose

One menacing emotion of midlife is the sense of stagnation at times associated with actually mastering a craft or profession. Erik Erikson's life stage of generativity vs. stagnation represents the mature conflict of adulthood and coincides with the idea of mid-life crisis. Generativity involves guiding the next generation and the concept is meant to include productivity and creativity. In a healthy family, parents show generativity through interest and care for their children. In an organization, leaders with generativity are able to better care for both the mission and their employees [10]. Success leads to feelings of usefulness and accomplishment, while failure results in shallow involvement in the world.

By failing to find a way to contribute, one may become stagnant and feel unproductive. These individuals may feel disconnected or uninvolved with their community and with society as a whole.

Managing Uncertainty

Individuals in midlife may experience both physical and psychological changes, including changing physical appearance, decreased stamina, loss of family or friends, divorce, and altered vision. Midlife can be termed a bridge age with disruptions in or leveling of employment goals, children leaving home, and parents needing more care, as well as the realization that less time remains to fulfill dreams [11].

Anderson [12] compared the feelings associated with changes in physical appearance and stamina and facing mortality to the stages of mourning identified by Kubler-Ross. These stages, developed to describe the emotional reaction following a loss, include denial, anger, bargaining, depression, and acceptance [13]. Introspection guides individuals toward acceptance of changes in midlife thereby leading to positive change in relationships and employment [14]. Inability to accept mistakes made while managing difficult situations, focusing on too many aspects of difficult situations, and demoralization from personal failures can amount to difficulty coping with changes in midlife [8].

Spirituality

Spirituality is important to a large percentage of the older adult population and serves as a promoter of healthy aging. A growing body of literature suggests that people often turn to religion when coping with stressful events. However, studies on the efficacy of religious coping for people dealing with stressful situations have yielded mixed results [15]. The study on resilience in midlife by McGinnis also concluded that many participants considered religious and/or spiritual strategies to be an important component of managing challenges; however, for a subset, spiritual strategies were not necessarily effective [8]. Spirituality is also a strength of character identified by Seligman and Peterson and along with the appreciation of beauty, gratitude, hope, and humor, constitute the virtue of Transcendence.

Emotional Objectivity

An individual's ability to set aside emotional reactions when dealing with distressing situations and solving life's problems also contributes toward resilience in midlife. This is in line with the emotional dysregulation model for anxiety and mood disorders, which views such disorders as the result of emotional dysregulation of negative emotions coupled with deficiencies in positive affect [16].

Adversity Level

Current literature suggests that early traumatic experiences and early childhood adversity (ECA) have long-lasting repercussions influencing our health as adults, incidence of

chronic disease, and quality of life indicators [17]. Life satisfaction is a common outcome of interest in research about successful aging [18, 19]. Life satisfaction involves an assessment of how well desired goals and actual outcomes have matched. Previous studies have established the effect of cumulative adversity, which included childhood experiences as well as adult adverse experiences, on life satisfaction [20, 21]. Life satisfaction is negatively associated in particular with early adversity [22, 23].

In addition, individuals with low socioeconomic status (SES) are at risk and vulnerable to accelerated aging. Yet, the biggest advantages with psychosocial resources and adaptive behaviors have been found for those at greatest risk, i.e., low SES couples with ECA [24]. "Differential susceptibility" refers to individual differences in the response to adversity (National Institute on Aging, 2012) [25]. It suggests that the same attributes that make an individual particularly sensitive to adversity may also make him or her more responsive to supportive interventions designed to offset the effects of adversity [5].

Humor

Humor plays an important role in resilience toward stress and trauma. Increased humor can contribute to the enhancement of positive life experiences, and lead to greater positive affect and psychological well-being [26]. Humor is also a character strength that contributes strongly to life satisfaction [27] and another strength of character component of the virtue of Transcendence. However, a major concern with some of the positive psychology work on humor and resiliency is the exclusive focus on a singular positive construct of humor. In contrast, the humor styles model recognizes not only the positive or adaptive aspects of humor, but also the maladaptive or negative aspects. Self-enhancing or affiliative humor can yield the expected resiliency effects, whereas self-defeating or aggressive humor may not [28, 29].

Changing Philosophy

With each stage of development our responsibilities continue to evolve and change. One's ability to adapt to these changes and demands of life and society affects one's life satisfaction. Individuals may experience restlessness as they evaluate their goals in earlier years and compare them to accomplishments, leading to a crisis. Those who remain in a stable personal and professional environment are able to derive greater satisfaction as the meaning of work and their role shifts, and the concept of generativity becomes noticeable [14].

One important predictor of well-being in midlife is education—where years of education acquired before age 25 has a lingering effect in helping foster a lifelong learning pattern along with better management of adverse life events in midlife [8].

Time volunteering and charitable donations in midlife seem to have a significant positive effect in psychological well-being later in life [30].

Conclusion

There is some evidence that stresses involving multiple role demands, or financial pressures, may cluster in midlife or take a greater toll in middle age [31]. Yet still today, there are many misconceptions about midlife, with the most common myth centered on the midlife crisis.

The stereotype of midlife crisis as portrayed in the media is depicted as a middle-aged man purchasing a sports car or acquiring a new hair color or style in an effort to defy the aging process. In reality, midlife is a critical juncture of growth and decline pathways. The salient issues of midlife involve balancing work and family responsibilities and demands in the midst of physical and psychological changes associated with aging.

The MIDUS survey shows that a crisis is not a typical midlife phenomenon. In a recent study in the United Kingdom, the reports of crises were higher, in the 40–60% range, although the incidence was comparable across adulthood [32]. About the same number said they experienced crises at other points in life. Of those who said

they had a midlife crisis, about half said it involved inner turmoil or angst associated with getting older. For the rest, it was tied to events such as divorce, job loss, or health problems, which can occur at any period [33]. Those who did experience a crisis in midlife were usually those who had upheavals at other times in their lives, and these individuals seemed to be driven more by a neurotic personality than advancing age [34, 35].

On the positive side, the MIDUS study found that those who had supportive social relationships, exercised regularly, and had positive attitudes about control in midlife were better able to maintain their functional health and cognitive skills over a 10-year period, and more of these positive factors were better [36].

Midlife also is a prime period for connections across earlier and later periods of the life course. Despite its challenges, midlife is a time to learn from stressful experiences, find meaning, and experience growth in the face of adversity.

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Professional Well-Being

6

Victoria Walden-Flynn and Erick Messias

Introduction

Most people in the world will spend a third of their adult lives at work, making well-being at work a key component of their overall well-being [1]. Positive interventions hold significant promise in helping prevent burnout and promote human flourishing both in the workplace and in general. Positive psychology and psychiatry are approaches to mental health, rather than mental illness, with an emphasis on enhancement of positive psychosocial traits, promotion of positive change, and development of well-being [2–5]. The focus on one's mental health, well-being, and both personal and professional success involves further developing positive coping, best practices, and shifting to this positive approach.

Transitions throughout training, job search, and a career can be both exciting and challenging, with the potential for self-doubt and imposter syndrome, burnout, turnover, and early career transition. While there are many exciting opportunities, there are also new responsibilities and challenges that may seem both daunting and insurmountable to young professionals. These challenges are more easily navigable by fostering specific virtues in each phase of professional development, from initial curiosity and optimism to informed perspective and gratitude, and ultimately to an appreciation of excellence in one's late career. Professional burnout is another challenge for professionals at each stage of their career. Professional burnout is a syndrome of emotional exhaustion, depersonalization or cynicism, and low sense of personal accomplishment in response to chronic occupational stressors [6]. Professional burnout affects one's ability to successfully navigate the unique challenges at various stages of professional development, especially those during early periods of role transition from learner to apprentice to early career professional. This is an increasing concern for human service professions, including first responders, teachers, and healthcare providers where burnout affects one's ability to find success and longevity in their profession. Professional burnout has become a profound concern for the institution of healthcare, as rates of burnout exceed 50% in students, trainees, and early career professionals [7–13].

With the guidance and direction of a professional coach, these transitions throughout one's professional development may be more easily traversed without these negative consequences

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_6

[14, 15]. The International Coach Federation (ICF) defines coaching as "partnering with clients in a thought-provoking and creative process that inspires them to maximize their personal and professional potential" [16]. A positive psychiatry approach, with an emphasis on further developing one's strengths and the positive psychosocial factors of optimism for personal mastery, perceived self-efficacy, consciousness, social engagement, and resilience, may help encourage and guide young professionals through their career and shift from an emphasis on preventing burnout to promoting wellness. Research using the Positive Psychotherapy framework, after N. Peseschkian, pointed to the capabilities of hope, love, time, and trust as particularly important to achieving life's priorities [17]

Key Positive Psychological Traits

Positive psychosocial factors (PPSF) include positive social and environmental influences as well as internal positive psychological traits (PPT). PPTs originate from the positive psychology movement which focuses on circumstances and processes that yield successful and optimal functioning in individuals, groups, and institutions. Enhancement of PPTs is necessary for both personal and professional development. While most PPTs are beneficial for personal growth, we will focus on a select few proposed to be most beneficial throughout various stages of one's professional and career development (Table 6.1). Figure 6.1 shows these key character traits and virtues, alongside opportunities and challenges faced at progressive stages of development in one's career.

Erik Erikson, a prominent psychoanalyst of the twentieth century, formulated a psychosocial development model identifying developmental stages throughout the lifespan. This model illustrates sequential development and identity formation [18]. His theory has primarily been used as a model for the development of children and adolescents. Darling-Fisher has focused on its application to adulthood and has shown that mastery of late developmental crises (generativity vs. stagnation and ego integrity vs. despair) in critical developmental periods is largely affected by advancing age [19, 20]. Here, the changes and challenges of professional development throughout a career from learner to retiree are likened to these developmental stages from childhood to older age, with advancing age and experience assisting in mastering these crises. There are various challenges and opportunities at each phase of one's career that when surpassed lead to further growth and mastery. In the above figure and in the sections to follow a proposed model is provided for pairing Erikson's psychosocial stages of development to phases of career development

Table 6.1 Opportunities, challenges, and key character traits throughout stages of development in a professional career

Stage of career	Opportunities and challenges	Erikson developmental phase	Associated virtue
Student	Knowledge acquisition; test-based performance evaluation	Autonomy vs. shame and doubt	Curiosity and optimism
Apprenticeship	Learning to apply acquired knowledge; supervised application of acquired knowledge; skill-based performance evaluation	Industry vs. inferiority	Love of learning
Early career	Transition to young professional; independent application of knowledge; skill-based performance evaluation	Professional identity vs. role confusion	Vitality and zest
Mid-career	Professional mastery and wisdom; leadership and mentorship development	Intimacy vs. isolation	Social and emotional intelligence
Late career	Expert professional mastery; knowledge and skill transfer to trainees	Generativity vs. stagnation	Perspective and gratitude
Retirement	Transition from professional life; evaluation of personal and professional life accomplishments	Integrity vs. ego despair	Appreciation of beauty and excellence



Fig. 6.1 Opportunities, challenges, and key character traits throughout a professional career development

with associated opportunities and challenges and specific character traits and virtues identified to foster.

Curiosity and Optimism

Point in career	Developmental stage	Character trait or virtue
Student and trainee	Autonomy vs. shame and doubt	Curiosity and optimism

Curiosity and optimism are important aspects of nearly every stage of one's professional development, especially those periods with prominent growth and transition. In the learner and trainee stages, an individual is struggling with the developmental challenge of knowledge acquisition and attempting to overcome self-doubt and increase autonomy. Peterson and Seligman describe curiosity as a positive emotional-motivational state where individuals "initiate and sustain goal-directed behaviors in response to incentive cues" [21]. Those in the earliest stages of their profession experience learning curves where large amounts of knowledge and skill must be acquired in order to ready oneself for future stages of one's career. An increased level of curiosity and optimism may serve students and trainees in knowledge acquisition and application. In fact, meta-analyses show curiosity accounts for 10% of the variance of learning and performance [22] with greater curiosity associated with greater learning, engagement, and performance [23]. Fostering the curiosity of students and trainees may keep them interested and engaged in the field for further occupational pursuits.

Optimism reflects the extent to which individuals expect favorable outcomes to occur and continue their effort toward obtaining goals and desirable outcomes [21]. Individuals with high levels of optimism tend to attribute positive outcomes to internal, stable factors and negative outcomes to external, temporary factors [2]. Optimism is an important trait for professional development and growth mindset as individuals with higher levels of optimism may be more likely to take on challenges, tackle new problems, and collaborate with others. Duckworth, a proponent of positive and well-being education, is dedicated to teaching character development and describes "grit" as the most reliable predictor of success. Grit, which she defines as passion and perseverance, is closely associated with Seligman's early studies on optimism [24]. In Seligman's research on depression, learned helplessness was viewed as the belief that one's actions did not influence outcomes. Grit and optimism are quite the opposite with an emphasis on internal motivators and influences.

Optimism and hope are essential for the ability to think about different possibilities and find the best course of achieving a more desirable future [3]. In their 2018 Positive Psychotherapy Clinician Manual, Rashid and Seligman describe a systematic process for building hope and optimism through their "Doors Opening" worksheet which illuminates optimism or pessimism through one's explanation for missed opportunities and "doors closing." An individual who describes the process of one door closing as external, temporary, and situationally dependent (rather than personalized, permanent, and pervasive) has increased optimism and more adaptive functioning in the wake of negative consequences. By pairing this activity with reflection and discussion with a professional coach,

therapist or mentor, unhelpful and overly critical perceptions can be altered for progression toward a more optimistic and flexible mindset considering work [3].

Love of Learning

Point in career	Developmental stage	Character trait or virtue
Apprenticeship	Industry vs. inferiority	Love of learning

Lifelong learners, with their zest for advancement, individually excel as well as profoundly influence their surrounding community. These lifelong learners are cognitively engaged and have a love of learning that is described as a "way in which a person engages new information and skills generally and/or the well-developed individual interest with which he or she engages a particular interest" [21]. These individuals typically have positive feelings regarding learning new material and acquiring new skills [25]. They are the individuals who "learn for the sake of learning" rather than for an immediate or extrinsic motivation [21]. Individuals in the apprenticeship phases of their career transition from learning material to learning the application of this material to various occupational situations. In this phase of development, an individual is striving for industry but is at risk for feelings of inferiority. Peterson and Seligman suggest individuals with love of learning as a strength would strongly endorse statements that suggest they "like to learn new things" and that despite current difficulty or inability to perform tasks they have confidence that they will be able to master the material and/or skills in the future. These individuals are able to "self-regulate efforts to persevere, despite challenge and frustration" [21]. This skill is becoming increasingly beneficial for those life-long learners in nearly every field—healthcare, business, education, finance—as advancements in technology rapidly change processes of day-to-day operations as well as communication avenues for exchange of information.

Love of learning is described as a character strength with strong intrinsic motivation. Even without love of learning as a primary strength, individuals can foster its development after gaining specific interests. The process of developing love of learning as a strength as one develops specific interests and has increased opportunities to further develop knowledge in this area leads an individual to gain value from learning by increased confidence, competence, and an optimistic sense of possibility [25]. Peterson and Seligman emphasize the roles of teachers and parents in supporting children in building a love of learning. In the professional domain, a coach or mentor may support an individual by connecting them to various topics of interest as well as encouraging them through periods of challenge and frustration.

Vitality and Zest

Point in career	Developmental stage	Character trait or virtue
Early career	Professional identity vs. role confusion	Vitality and Zest

A vital person infectiously energizes those with whom they come into contact [21]. The word "vitality" is derived from vita, or life, and one who is vital can be described as having vigor, enthusiasm, and spirit. This vitality, or life force, is a positive energy from within that energizes and motivates the self and others. The concept of vitality is far from new, with origins in ancient Eastern philosophies and healing practices. The Chinese describe *chi*, a vital force or energy that is the source of life, creativity, right action, and harmony. The Japanese concept of *Ki*, or energy and power one can draw upon, is related to physical, emotional, and mental health. Balinese healers work to bring together *bayu*, a vital spiritual or life force underlying growth and resistance to illness [26]. Each of these descriptions emphasizes the role of vitality in holistic growth and health. Ryan and Frederick developed a measure of subjective vitality, or a positive feeling of aliveness and fullness of energy which can identify differences between individuals' report of vitality as well as changes within an individual over time given various situations and factors

[26]. Early career professionals focus on the development of professional interests and a professional identity and once this identity is formed their enthusiasm and vitality is nearly palpable.

Individuals who are vital, or full of life, are internally motivated for positive influence and change. Several studies have attempted to measure and or enhance vitality. Nix et al. showed that while success in completing tasks can lead to happiness, only autonomous tasks maintain or enhance vitality [27]. Thus, those activities that are self-motivated and conclude with a perceived internal locus of causality have a greater influence on vitality. Exercise and a healthy lifestyle have been shown to improve vitality [28]. Jakobson et al. showed that physical exercise, performed with colleagues during working hours, is more effective than other exercises at improving vitality at work [29]. Interestingly, when individuals are asked to monitor and report the amount of time they spend sitting in a day, over time they report significant decreases in sitting time and significant increases in vitality at work scores [30]. Hendrikson and collaborators' Workplace Health Promotion Program which included training, workshops, and individual coaching significantly improved workplace vitality, performance, self-management, and sickness leave [31]. Ryan et al. have experimentally shown that contact with nature positively affects vitality [32]. In these studies, vignettes describing being in nature, 15-minute walks in nature, and being shown photographs of nature scenes all increase vitality. This illustrates that any nature-related sensory input—auditory, physical touch, and visual—enhances an individual's sense of vitality. Additionally, a sense of autonomy and increased social contacts improves vitality [33, 34]. Workplace vitality may be increased by incorporation of several of the previously mentioned interventions—schedule informal meetings as walking meetings to increase exercise and reduce sitting time; have lunch meetings or work gatherings in outdoor relaxation or garden spaces; have team building activities outdoors to include a nature component; and allow team members to work on committees or projects they find passion for.

Social and Emotional Intelligence

Point in career	Developmental stage	Character trait or virtue
Mid-career	Intimacy vs. isolation	Social and emotional intelligence

Intelligence, the ability to think abstractly, can be divided into cognitive intelligences and the *hot intelligences* which encompass signals concerning motives, feelings, and relationships [21]. Social and emotional intelligence are two of the hot intelligences and individuals with high social and emotional intelligence are skilled in: identifying emotion in faces, voices, and designs; using emotional information to facilitate cognitive activities; understanding what emotions mean in various relationships; managing and regulating emotions of the self and others; using social information to facilitate group cooperation; and identifying social relationships and hierarchies among groups [21]. Individuals in the midpoint of their career have successfully traversed the initial challenges and transitions of a career. These individuals focus on fostering relationships with trainees through coaching or mentoring and relationships with peers through membership and leadership in local, national, and global organizations. Goleman argues successful, effective leaders stand out because of their emotional intelligence. Emotional intelligence is valued equally, if not more so, than IQ in his concept of successful leaders [35, 36].

Emotional intelligence involves the basic skills of self-awareness, self-regulation, motivation, empathy, and social skill. These skills can be strengthened through practice and feedback from colleagues or coaches. David and Congleton have worked with leaders in various industries to build emotional agility, an emotional intelligence skill which enables people to approach their inner experiences in a mindful, values driven way rather than attempting to ignore, suppress, or alter these thoughts and emotions. They encourage four practices, based on Acceptance and Commitment Therapy, which include recognition of thought and emotion patterns, labeling of thoughts and emotions, accepting ideas and emo-

tions with an open attitude, and acting based on individual and organizational values. Strengthening emotional agility and social-emotional intelligence can alleviate stress, reduce errors, increase innovation, and improve performance [35, 37].

Perspective and Gratitude

Point in career	Developmental stage	Character trait or virtue
Late career	Generativity vs. stagnation	Perspective, kindness, gratitude

Gratitude is a sense of thankfulness and joy in response to receiving a gift. Individuals late in their career may look back on their professional life and reflect on how others have positively influenced their professional development by investing time and energy into their success. A sense of gratitude arises from the belief that one has benefited because of the actions of another person, the acknowledgement of this gift of investment, and appreciation and recognition of the value of that gift [21]. Late career individuals who have benefited from the assistance of another person often work to give back to other trainee and early career individuals. Their generativity and inclusion positively affects the development of those trainees and early career professionals by fostering many of virtues mentioned above.

Gratitude interventions often include a daily gratitude journal in which an individual reflects on people or experiences for which they are grateful. Cheng et al. showed that just two work-related diary entries per week decrease perceived stress at work [38]. Stegen and Wankier illustrated that generating gratitude in the workplace through various gratitude interventions increases job satisfaction [39]. Seligman showed that even a one-time letter of gratitude written and delivered sustains significant decreases in depressive symptoms and increases happiness ratings for 4 weeks [4]. Increasing opportunities for gratitude may positively affect the individual, the community at work, and work performance.

Appreciation of Beauty and Excellence

Point in career	Developmental stage	Character trait or virtue
Retirement	Integrity vs. ego despair	Appreciation of beauty and excellence

Individuals with an appreciation of beauty experience self-transcendent emotions like awe and admiration when encountering perceived beauty and excellence in their surroundings. Aesthetic sensitivity and responsiveness allow one to more fully appreciate the world around them and to forget one's own concerns and attachments. Appreciation, when associated with a loss of ego and openness to others, enables self-transcendence [21]. Individuals who have reached the later phases of their professional career and retirement theoretically reach what Maslow would describe as the "peak experience" of one's career. These peak experiences capitalize on a gained appreciation and include a selfless attitude, receptive and humble cognition, enhanced ability to see, hear, and connect with others, and a view of the world as good and beautiful despite conflicts and suffering [40]. An individual in the retirement phase of their career, who has reached the developmental phase of self-integrity, sees the workplace as "all of a piece and that one has his place in it" [40].

The developmental course of appreciation remains unclear. It is thought to be highly heritable and that openness to experience plays a role [21]. Searching for excellence in others and modeling that excellence throughout one's career may help one better appreciate the excellence around them. Fryer-Edwards et al. took advantage of this notion in their Committee on Continuous Professionalism Improvement program which emphasizes appreciative inquiry and celebrates merit in their community, with the goal of further increasing positivity and excellence [41]. This positive inquiry may enhance one's ego integrity as they transition from a role with direct influence on their chosen career to a reflection on the course of their career and indirect influence on their younger colleagues.

The Problem of Professional Burnout

Individuals often state their motivation for entering healthcare, human service, and educational fields is to connect with others, make a difference, and improve the lives of others. This empathic approach energizes individuals to become fully invested in their clients and work goals, often working long hours giving up personal time in order to assist with a student or client in need. Higher levels of humanistic characteristics—generosity, compassion, altruism, social and emotional intelligence—are important for fostering interpersonal relationships [21]. Unfortunately, a higher level of these humanistic characteristics, specifically empathy, has also been associated with increased levels of professional burnout [42]. Professional burnout is a psychological syndrome of emotional exhaustion, depersonalization or cynicism, and a low sense of personal accomplishment or lack of professional efficacy in response to chronic interpersonal and occupational stressors [6, 43–45]. Due to the emotional and systemic/administrative demands of the profession, human service professionals may be at an increased risk for burnout.

The development and sequelae of burnout is not a universal process, and individuals are not uniformly affected [46]. Various conceptual models have been created to describe the process, most of which involve imbalances between individuals and their workplace or job demands [44]. The transactional model of burnout describes sequential stages of stressors with individual and job imbalances—from an imbalance in job demands and individual capabilities, to individual strain, and finally defensive coping manifested as disengagement or cynicism [47]. This model describes a cause and effect process where increased stress leads to increased strain, similar to an elastic stretched thin. The Job Demands-Resources Model and Conservation of Resources models describe a reactionary process to persistent job demands. In these models, unrelenting demands and stressors lead to either liquidation of resources or the perception of impending loss of resources and a resultant attempt to maintain the remaining resources (e.g., time, energy, etc.) [48, 49]. These conceptual models of burnout are similar to a car with an empty gas tank. The Areas of Worklife model identifies six key areas in which there are person-job mismatches leading to burnout. These six areas include workload, control, reward, community, fairness, and values [50]. In this model, a mismatch between goals of an individual and that of the workplace in one or more of these areas leads to burnout. Interestingly, rewards and compensation are helpful in keeping symptoms of disengagement and emotional exhaustion at bay but monetary rewards are not the most incentivizing strategy to increase productivity and enjoyment at work [48]. Our own research pointed to two main factors associated with professional burnout in an academic medical center: job control and meaning at work [51].

Significant time and energy have been invested in identifying an individual's burnout, an organizational prevalence, and key influences because there are very real consequences to this phenomenon. In the healthcare field, burnout has been associated with professional consequences of increased medical errors, decreased quality of care, reduced patient satisfaction, and decreased patient compliance with treatment plans [11]. Additionally, personal consequences of burnout include broken relationships, maladaptive coping (e.g., alcohol and substance use), depression, and suicide [11]. Several studies have examined the relationship between burnout and depression [52–54], with the current understanding that there is a complex relationship between these distinct entities [44, 55–58]. Burnout has an identified negative association with professionalism, altruism, and what often led individuals to the human service fields, their sense of calling [59, 60]. The financial well-being of an organization is also affected as burnout is associated with the expressed intent to leave one's job, higher rates of turnover, and productivity losses [12]. These professional and personal consequences of burnout are of concern for patients, providers, and the institution of medicine.

Interventions

Given the many consequences of professional burnout, there are multiple budding solutions for reducing professional burnout, increasing employee engagement, and improving workplace well-being [11, 13, 61–65]. Interventions have been shown to reduce overall burnout modestly, some studies estimating reductions of 10% [13]. These interventions can be classified as individual, organizational, or leadership-based (Fig. 6.2). Interventions must be developed to meet the needs of the individuals and organizations involved rather than application of a universal, one-size-fits-all all solution [11].

At the individual level, common interventions include mindfulness training, meditation, gratitude exercises, or reengagement with hobbies [38, 39, 55–58, 66, 67]. Significant improvements in perceived stress and professional satisfaction are gained by reflecting on one's thoughts, emotions, and biases during challenging interpersonal encounters [68]. Small group curriculum on mindfulness, reflection, and shared experience leads to statistically significant and sustained improvements in feelings of empowerment and engagement at work. It also has significant improvements in finding meaning, emotional exhaustion, depersonalization, and overall burnout [63]. Interventions focused on improved communication increase satisfaction and decrease intent to leave one's job while interventions focused on workflow changes and targeted quality improvement reduce overall burnout [62].

At the organizational level interventions often involve workflow restructuring and improving workplace practices, providing occupational training, and developing workshops to reduce mental stress. Individuals with minimal, or no, burnout often participate in these programs but individuals with high degrees of burnout have been shown to be less likely to voluntarily participate in these organization-driven interventions [64]. Shanafelt et al. suggest nine organizational interventions to promote well-being. This list includes (1) acknowledging stress

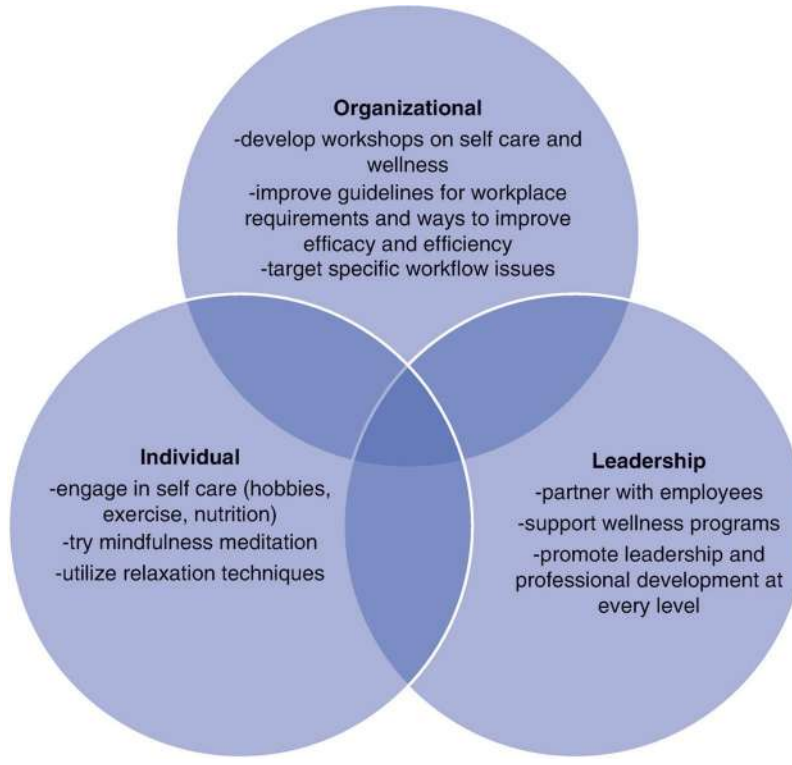


Fig. 6.2 Organizational-, leadership-, and individual-based interventions to improve engagement and performance and prevent professional burnout

and burnout with leadership-driven open dialogue and assessments of burnout over time; (2) harnessing the power of leadership to enhance leadership skills and to recognize the unique talents and motivations of members of their team; (3) developing and implementing interventions which target-specific driver mismatches in individual work units; (4) cultivating community at work through debriefing after errors, providing feedback, and celebrating achievements; (5) using caution with productivity incentives that may encourage decreased quality of care (shorter appointments, increased workup, etc.) or increased work hours leaving employees susceptible to burnout; (6) aligning values where the team and organization are working toward a common goal and improve community at work; (7) promoting flexibility with work schedules and encouraging work-life integration; (8) providing resources to promote resilience (skills training, narrative medicine, mindfulness, positive psychology exercises) and self-care (exercise, diet, sleep, financial health, relationships, hobbies); and (9) facilitating and funding organizational science [11]. These nine interventions target nearly every one of their proposed drivers for engagement or burnout in work—values and culture, meaning, flexibility, community, resources, workload, and work-life integration. Other programs suggest addressing the psychological needs of employees, cultivating organization-employee relationships, and sponsoring leadership development in order to develop comradery and excellence https://www.merriam-webster.com/dictionary/comradery [69].

Executive Coaching Approach to Professional Development

Professional coaching is a results-oriented method of "enhancing self-awareness, drawing on individual strengths, questioning self-defeating thoughts and beliefs, examining new perspectives, and aligning personal values with professional duties" [70]. As described above, the International Coaching Federation (ICF) defines the goal of coaching as partnering with a client and inspiring them to reach their full potential through a thought-provoking, question-driven, creative process [16]. Despite sharing attributes with mentoring, therapy, and consulting, coaching has specific goals, methods, and training making it a separate category of intervention that is suitable for professional development—see Table 6.2 for the differences between these modalities. Professional coaches engage clients with their present challenges and encourage them to think of alternative perspectives and self-identified action-oriented solutions. This process ultimately assists clients in the self-directed process of achieving their goals [14, 15, 70, 71].

A professional coach is most often confused with the role of a mentor. A mentor has a desire to

Table 6.2 Differences between coaching, consulting, mentorship, training, and therapy modalities

Modality	Coaching	Consulting	Mentoring	Training	Therapy
Goal	Achieving goals and reaching full potential	Implementing programs	Modeling	Knowledge transfer and acquisition	Managing symptoms and relapse prevention
Expertise	Coaching	Subject matter experts	Experience	Implementation	Psychotherapy modalities
Focus	Present goals, consistent directed action, future results	Past processes applied for future outcomes	Past successes modeled for present success	Past knowledge applied to present circumstances	Past determinants and current symptoms
Techniques	Powerful questioning, client-directed exercises, challenges	Observation, implementation, testing	Advising, modeling	Case studies, skill building, systems, planning	Vary from supportive, to behavioral, to insight-oriented
Services	Pragmatism, accountability	Proven methods for success	Proven path for success	Practical strategic plans	Group and individual sessions

share knowledge and experience and develops a personal and professional relationship with a mentee. Mentors are described as having multiple roles: advisor with career experience and desire to share knowledge; supporter who gives emotional and moral encouragement; tutor who gives specific feedback; sponsor who shares information and aids in opportunities for growth; and a model who demonstrates their own as well as guides the development of their mentees professional identity [72].

According to the ICF, coaching has several core competencies which range from establishing an agreement, being an active and present listener, engaging powerful questioning and maintaining communication, assisting with goal setting, and holding clients accountable [15, 16]. The core competencies, according to the ICF, are listed in Table 6.3.

Different coaching certifications require a variety of standards, training, supervision, and practice. Several tools applied in coaching have their origins in positive psychology, neurosciences, and traditional forms of psychotherapy, in particular Cognitive Behavioral Therapy (CBT) [70, 71, 73]. Positive psychiatry's emphasis on expanding on strengths and developing positive psychological traits (PPT) align nicely with the core competencies of coaching. PPTs focus on attributes that determine healthy outcomes and greater functioning in the face of normative transitions and adversities. PPTs of personal mastery, perceived self-efficacy, and resilience are significant for coaching through one's professional development [7]. Developing and nurturing PPTs in a professional coaching relationship may lead to positive attitudes, increased work effort, and ultimately professional success. As a coach, it is usually more productive to focus on a client's strengths rather than weaknesses and to enhance skills and behaviors that have previously been helpful for a client [21]. Per Polly and Britton's executive coach case description in the recently published Positive Psychiatry: A Casebook, coaches who draw on positive psychology often emphasize client's strengths, growth mindsets, and self-efficacy [56].

Table 6.3 International Coaching Federation core coaching competencies

Coaching core competencies
Meeting ethical guidelines and professional standards
Establishing the coaching agreement
Establishing trust and intimacy with the client
Having coaching presence
Exercising active listening
Engaging in powerful questioning
Maintaining direct communication
Creating awareness
Designing actions
Setting plans and goals
Managing progress and providing accountability

Positive psychology coaching uses one's strengths to emphasize engagement, meaning, and accomplishment [70]. Palamara et al. created a strengths-based coaching program for resident physicians with the goal of establishing a safe environment for support and guidance through internship. Coaches were trained in positive psychology exercises that guided each session—including the Best Reflective Self, Values in Action Signature Strengths Survey, GROW (goal, reality, options, way forward), and PERMA (positive emotions, engagement, relationships, meaning, accomplishment). At the conclusion of the program, 96% of interns reported they either definitely or probably would recommend this coaching program. Interns also reported statistically significant improvements in reported opportunities to reflect on performance and communication with their coach. While not statistically significant, there were lower reports of emotional exhaustion, a component of resident physician burnout [58].

As previously discussed, professional burnout is a syndrome of emotional exhaustion, cynicism, and depersonalization, and a decreased sense of personal accomplishment [6, 43–45]. Coaches who emphasize enhanced self-reflection and awareness can improve a client's flexibility and resilience in the face of workplace

stressors [74]. In fact, studies have suggested fostering this self-awareness may help identify one's values and find meaning in their work [63]. West et al. capitalized on this by designing facilitators to lead small group discussions on mindfulness, reflection, shared experience, and group learning. They explored topics relevant to the physician work experience, including meaning in work, balance, mistakes, community, and caring for patients. The results of this work showed improvement in empowerment and engagement as well as decreased rates of depersonalization, both of which were sustained at 1 year [63].

Professional coaching is an emerging approach for the professional development of medical practitioners [63, 75]. Examples of dialogue between a client and coach can be reviewed to gain insight into the potential client-coach experience [70]. By increasing engagement at work and inspiring recovery of personal interests, coaches encourage resilience and a sense of meaning [70]. There are many potential positive results of professional coaching including improved self-awareness, insight-oriented behavioral changes, decreased burnout, increased resilience, retention, and engagement in work and community.

Summary

Most adults spend the majority of their time at work, making well-being at work a major factor in overall well-being. Professional burnout affects one's ability to maintain engagement and productivity at work as well as successfully navigate the unique challenges at various stages of one's career. There are both personal and professional consequences to burnout, and unfortunately the rate of burnout in human service professions is increasing. An emphasis on positive psychosocial factors and enhancement of various character strengths and virtues may counter the negative response to burnout and professional development challenges. With the help of a professional coach or mentor, professional development can be navigated for personal, professional, and leadership success.

Key Points

- Individuals face unique challenges and opportunities as they progress through various stages of their profession.
- These challenges can be viewed through a developmental lens, similar to Erikson's psychosocial development model, and traversed by enhancement of various character strengths and virtues.
- Professional burnout is a threat to today's organizations and society at large. With increasing emotional exhaustion, depersonalization, and decreasing self-efficacy come personal and professional consequences that affect work-life satisfaction and workforce sustainability and turnover.
- Interventions for improving satisfaction and reducing burnout by promoting wellness show promise. These interventions may be at individual, organizational, or leadership levels and should target the specific needs of individuals involved.
- With the help of a professional coach or mentor, professional developmental challenges can be navigated and personal, professional, and leadership development successfully guided.

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Successful Aging

7

Paulette Mehta, Carter Starnes, Erick Messias, and Noam Grysman

The greatest thing in life is to die young as late as possible. –George Bernard Shaw

Introduction

People are aging in the United States (U.S.) and around the world. Whereas the average life expectancy in the U.S. was 47 years in 1900, it is 74.8 years for men and 80.2 years for women as of 2022 [1]. In 1900, the number of people over 65 years of age was 1 in 25, now it is 1 in 8. Furthermore, the population over 85 years is now the fastest-growing segment of the American population. The number of people over 65 years of age was three million in 1994 and is expected to be 19 million by 2050. The causes of morbidity and mortality have also changed from primarily infectious disease in the early 1900s (pneumonia and tuberculosis) to chronic diseases for which the underlying causes are now primarily psychosocial problems, i.e., obesity, smoking, physical inactivity, poor diet, addictions (opioid and others substances), suicide, and homicide [2]. These changes in demographics and in causes of disease will change public health needs and offer opportunities for interventions for decades to come. Psychiatrists will have an important role in shaping response to this public health upheaval and in promoting techniques for successful aging for patients with and at risk for mental disorders, for responding to the underlying psychosocial issues of our time, and for expanding and disseminating the wisdom of elders to the young.

Normal aging is usually associated with some decline in physical functioning and health while pathological aging is associated with premature decline in physical functioning, disease, and disability. In contrast, successful aging has been generally considered as living a long life with minimal disability and disease, with good function and social engagement, although its precise definition is still elusive [3].

There are many longitudinal studies on aging which began in the mid-1900s (see Table 7.1).

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_7

Table 7.1 Major longitudinal studies on aging

Year	Study name or cohort	Comments
1948	Framingham Heart Studies	Longest ongoing study of CV health, defined role of smoking, lipids, BP, psychosocial factors, family history, CHF, and genomics
1955	Duke Longitudinal Studies of Normal Aging	Evaluated cardiac, vascular, neurologic, psychosocial factors, and aging
1958	Baltimore Longitudinal Study of Aging	Showed variability of aging trajectories and relationships to CV disease and dementia
1961	Kansas City Study of Adult Life	Longitudinal study identifying aging trajectories and hierarchical goals
1976	Nurse Health Study	Studied married nurses from 11 states re birth control, smoking, cancer, CV disease, lifestyle factors, behaviors, and personal traits in relation to CV and >30 other diseases
1984	MacArthur Studies	Rowe and Kahn established the MacArthur studies and evaluated new biopsychosocial model of aging in 1000 elders; showed importance of social engagement, vitality, and resilience in successful aging
1991	Manitoba Elderly Study	Elders evaluated in 1971 and 1983, of >100 parameters, the only factors related to successful aging were age, 4 measures of health status, 2 of mental health and not having spouse die or enter nursing home
1993	MacArthur Studies	Using biopsychosocial model, successful aging related mostly to non-smoking, exercise, volunteer activity, and psychological health
1995	Nuns' study (Aging and Cognitive Function)	Long-term study of Roman Catholic nuns, evaluated physical, mental, cognitive, writing, and autopsy studies; found that education and social involvement could mitigate against early Alzheimer's disease symptoms
1995	Honolulu Asia Aging Study	Goal was to determine if there were differences in neuropathic disease and mortality in Japanese-American men in Hawaii. They identified 5 types of brain lesions independently associated with dementia
1998	Predictors of SA in Men with high life expectancies	Evaluated men of Japanese heritage; predictors of successful aging: Clinical disease, impairment
1999	Canadian Study of Health and Aging	Elders over 85 years; 79% felt well despite functional impairments
1999	Aging Successfully Until Death in Old Age	High physical activity associated with two-fold greater chance for dying without disability
1999	Berlin Study of Aging I (BASE-I)	Study showed variability of aging trajectories and relationships to CV disease and dementia
1999	New England Centenarian Study	Identified many new genetic variants associated with long life and resistance to diseases of aging
2000	SAGE Study (Successful AGing Evaluation study, UCSD)	UCSD, study of 1800 randomly selected, representative, community-dwelling people in San Diego CA focusing on cognitive and emotional factors
2001	CV Health Study	Predictors for successful aging were physical activity, non-smoking, optimal weight, and no uncontrolled diabetes, blood pressure, or lipid abnormalities
2001	Successful Aging Over a Lifetime	2 cohorts of boys with and without college education studied for 60 years or until death; identified 8 variables affecting survival, all controllable
2006	Alameda County Study	Evaluated elders born between 1895 and 1919, 20% healthy; successful aging related to race, family income, blood pressure, arthritis, back pain, smoking, weight, and alcohol
2010	NuAGE (Nutrition and Aging), Quebec	Evaluated 1741 women, 68–82 years for 3 years; decline was related to age, physical capacity, and depression

(continued)

Table 7.1 (continued)

Year	Study name or cohort	Comments
2011	MacArthur Studies	Evaluated 499 people from original cohort, APOE-E4 related to worse baseline cognitive function for which B vitamins did not help
2014	Women's Health Initiative	Very large study on women >80 years, physical function found to be related to demographics, psychosocial, behavioral health, and psychosocial support. Optimism mitigated depression
2014	Berlin Study of Aging I (BASE-II)	Expansion of BASE I study with additional 1600 people from 60 to 80 years and 600 younger subjects, 20–35 years; ongoing
2014	IDEAL Study (Insight into Determinants of Exceptional Aging and Longevity)	Recruiting subjects now for continuation of Baltimore longitudinal aging studies; will focus on aspects of successful aging in normal healthy subjects over 80 years of age
2015	SAGES Study (Successful Aging After Elective Surgery)	Study assesses long-term impact of delirium and other late effects after elective surgery in the aging
2015	Veterans Affairs Normative Aging Study	Assesses impact of cherished vs. hostile vs. ordinary child rearing on late life satisfaction and successful aging
2015	Uppsala Longitudinal Study of Adult Men	Assesses predictive factors for late-life independence. Optimal BMI and non-smoking are predictive
2015	National Health and Resilience in Veterans Study	Successful aging in veterans is related to health behaviors, social engagement, and disposition
2016	Longitudinal Aging Study (Amsterdam)	Large Dutch study of elders aged 55–85 years evaluated over 16 years. Successful aging was related to education, income, occupation, SES, and emotional resilience
2016	Vanderbilt Memory and Aging Project	Ongoing long-term evaluation of cardiovascular and brain health in people with mild cognitive impairment
2016	Women's Health Initiative	Most rated health as good or better despite disability/disease, often self-rated with high levels of resilience, self-control, and self-mastery
2017	ACHIEVE-P (Aging and Cognitive National Evaluation in Elders Pilot Program)	Pilot program to study the impact of hearing loss and cognitive decline in the elderly
2018	Occupational Determinants of Successful Aging in Older Physicians	Large study of occupational and personal aging in physicians which was related to age, gender, depression, cognitive state, anxiety, and work. Work centrality was very high and many physicians never retire possibly because they do not know what else to do
2020	The Health and Retirement Study (HRS)	Highlights the impact of cognitive decline on retirement and health outcomes. The 2020 wave of this study focuses on the effects of wealth disparities, demonstrating that economic inequality significantly influences health trajectories in older adults
Ongoing	The Harvard Study of Adult Development	The Harvard Study of Adult Development, ongoing for over 80 years, continues to provide valuable insights into aging. Recent findings emphasize the role of social connections in promoting healthy aging, showing that strong relationships and life satisfaction are key determinants of well-being in later life

Early studies were purely biomedical and focused on survival, causes of death, risk factors for disease, and protective influences mitigating the risks. Later studies added cognitive, behavioral, psychosocial, and economic factors. Some studies have focused on patients with psychiatric disease, mostly anxiety, depression, and schizophrenia. More recent studies are focusing on positive psychological and psychiatric factors such as optimism, resilience, and wisdom with

age. These qualities may take a lifetime to fully develop and may enrich one's personal life as well as the larger community and world.

Methodological Issues

Studies on aging share common obstacles, mostly lack of precise definitions. Depp, in his seminal review of aging literature, noted that there were 29 different definitions of successful aging in his review of 28 studies which had been published prior to 2006 [3]. Thus, successful aging may still be an elusive concept [4].

Similarly, there is not a single definition of elder age or how to distinguish biologically young-old from old-old, and oldest-old [5]. Although chronological age is easy to calculate, biological age (which considers not only the time elapsed, but also a number of different biological and physiological developmental factors, such as genetics, lifestyle, nutrition, and comorbidities [105]) is not, and biological age may be more important than chronological age in survival and in successful aging.

Another methodological problem is that subjects being studied and scientists studying them have different perceptions of any one person's quality of aging [6]. Elders almost always rate their aging as much better than do their investigators. One person's perception of success may be very different from another's. Successful aging, like beauty, may thus be in the eye of the beholder making any single definition non-generalizable.

Phelan and others were among the first to realize that elders often felt much better than their providers imagined [6]. The reason for this is that despite functional impairment and limitations, people can overcome such limitations with optimism, resilience, spirituality, and wisdom [7–10]. Although this is a methodological problem, it is also potentially a resource with which to discover methods of overcoming physical limitations allowing for more successful aging.

Thus, there is no single subjective or objective measure to define successful aging. A biomarker for age and for successful aging would be useful but is not available. Jeste et al. have suggested that telomere length may be a biomarker for age. This possibility is under investigation [11]. Other investigators have suggested that markers of chronic inflammation (e.g., IL-6, TNF-a, CRP, ESR) may be better markers [12]. Some investigators claim that radiologic studies, in particular brain fMRIs, PET, and SPECT scans, may be even better markers for brain health and early detection of degenerative diseases of the brain prior to clinical manifestations [13]. These scans then could diagnose brain disease early and help to begin experimental treatments at the first sign of radiologic detection, even before any clinical manifestation of disease.

Longitudinal Studies on Aging

From Biomedical Model to Biopsychosocial Economic Models

Table 7.1 lists key clinical trials but there are many other trials around the world. Early studies focused on lifespan, causes of morbidity and mortality, identification of risk factors for disease, and protective factors mitigating the risks. Early studies initially used a strictly biomedical model and emphasized disease and disability as limitations to successful aging. Gradually over time, this pure biomedical model evolved to incorporate cognitive, psychosocial, behavioral, and economic factors, creating a much more robust and comprehensive model.

The focus in aging research has consequently changed from a single disease focus to a more multimodal systemic approach [14]. Most recent aging studies are now organized by teams of specialists including geriatricians, internists, cardiologists, physiologists, psychologists, psychiatrists, and others. This is especially important since most age-related diseases involve multiple organs, although the underlying mechanisms of disease in each of these organs may be similar. Thus, factors which are dangerous for the heart, such as high blood pressure, overweight/obesity, smoking, excess cholesterol, and triglycerides, are also dangerous for the brain.

As attention has turned to a cognitive-behavioral-biopsychosocial-economic model, so

too has attention turned to the brain as a co-equal to the heart, which had previously taken center stage as a single focus of aging studies. This shift probably occurred due to a variety of reasons.

First, some of the most dangerous diseases of the 1960s were becoming more manageable and no longer needed the same single focus of attention. In particular, mortality from coronary artery disease began to decline in the 1980s [2] and beyond as a result of healthier lifestyles and more effective medications.

Second, it was clear that psychosocial factors modulated physical functioning such that patients were often quite happy and content despite physical limitations and despite their researchers' presumptions. These psychosocial parameters needed to be included in medical models to close the gap between subjects' experiences and researchers' assumptions.

Third, the 1990s became the "Decade of the Brain" which brought attention to the brain and its impact on health and aging, as well as increased national funding [15]. Many discoveries arose during this period of time including new generations of antidepressants and antipsychotics. More striking however was the discovery that the brain could heal itself and that brain growth and remodeling occurred throughout life. Remodeling could occur through the regeneration of tissue from endogenous brain stem cells, stimulated by brain-derived neurotropic and other growth factors. This was in direct contrast to the previously prevailing belief that development or regeneration of the brain could not occur after early adulthood [16]. This neuroplasticity could also result in rewiring of neural connections and thus trauma pathways could be mitigated, and regeneration could occur well into late life. Some qualities arising from the brain could even blossom at the end of life, notably resilience and wisdom, natural antidotes to anxiety and depression [17–19].

Fourth, it is possible that interest in psychosocial parameters in aging may be related to the coming of age of the baby boomer generation. Baby boomers like to control their destiny and to try novel therapies; moreover, they often had already experimented with psychedelic and psychotropic drugs and may have also leaned on psychologists and psychiatrists in stressful times. As such, they may have been more receptive to psychosocial studies and treatments. Of course, many baby boomer investigators were also coming of age, subject to the same dynamics and may have been more interested in studying psychosocial factors for the same reasons as the study subjects.

Thus, it is no surprise that cognitive, behavioral psychosocial-economic aspects became a part of the successful aging model as the first wave of longitudinal studies gave way to subsequent waves. Table 7.1 depicts many but not all of the major longitudinal aging studies from the mid-1900s until now. Below is a description of some of the more influential studies.

The Framingham Study

The largest of the early longitudinal studies is the Framingham study which started in 1948 and is still ongoing [20]. This study enrolled more than 5000 people between 30 and 62 years of age initially and has gradually expanded to involve three cohorts consisting of children of the original cohort as well as other cohorts with increasingly more diverse ethnic and racial composition. The emphasis from the very start was on the cardiovascular system especially coronary artery disease, congestive heart, and atherosclerosis.

More than a thousand papers have come out of this very long and distinguished study and form the basis of much of our current knowledge of risk factors for coronary artery disease. For example, investigators defined the role of hypertension, poor diet, lack of exercise, high blood pressure, diabetes mellitus, lipid abnormalities, and others in exacerbating heart disease. It also established the salutary effect of exercise, diet, optimal weight, and drugs such as aspirin and others.

Later studies in the Framingham Study focused on psychosocial findings and found, for example, that social contacts determine to a large extent whether an individual will become overweight/obese and that poor memory was more likely if their parents had dementia.

The Baltimore Longitudinal Study of Aging (BLSA)

Another very famous early and long-lasting longitudinal study is the Baltimore Longitudinal Study of Aging [21]. More than 3000 people between the ages of 60 and late 90s were entered into this study and were evaluated on a yearly basis for their aging trajectory. The major aims were to establish the types of aging trajectory and link them to physical and cognitive, mental, psycho-socio-economic factors. These studies had a focus on physical strength, energy expenditures with different types and intensities of physical activity, muscle strength, balance, bone mineralization, proprioception, and others. Brain imaging and PET scans were routinely done to identify the role of brain physiology in aging and to identify the earliest possible evidence of Alzheimer's disease.

A major feature of the study was comparing patients to themselves rather than comparing old to young people. The study generated more than 800 papers with several general conclusions that aging is very individualized, there is no specific pattern trajectory for all people, and that aging does not necessarily cause disease. However, people do lose muscle mass and muscle strength over time, and this can be partially overcome through vigorous physical activity. They also identified waist circumference with body mass index (BMI) as better than BMI alone for predicting coronary artery disease. They also showed that physical activity can prevent problems even when begun late in life, that balance exercises can reduce falls, that strength exercise can build muscles and prevent osteoporosis, and that flexibility exercises keep bodies limber.

The IDEAL Study

In 2014, the Baltimore Longitudinal Aging Study evolved into the IDEAL (Insight into Determinants of Exceptional Aging and Longevity) study [22]. This study sought to identify characteristics of successful or more-than-successful aging. Requirements for entry into the study are age over 80 years, good health, no major disease, no medications, normal cognitive function, and high physical activity. It is being conducted by the same investigators as those who conducted the Baltimore Longitudinal Aging Study. Study subjects are required to return every year for extensive studies, and results are pending.

The Nurses' Health Study

Another noteworthy early large longitudinal study focused on women is the Nurses' Health Study [23]. The Nurses' Health Study was started in 1976 and is now in its third generation of studies. Overall, more than 250,000 women have been a part of his study which has been continuously funded from the National Institutes of Health. Initially, the main purpose was to evaluate the long-term effects of oral contraceptives, but it has also evaluated many other aspects of health. Women who were married, registered nurses between 30 and 55 years of age, and who lived in one of 11 of the most populated states were invited to join the study. The study focused on contraceptive use, smoking, heart disease, cancer, lifestyle, psychosocial behaviors, and personal characteristics as related to more than 30 different diseases. Nutrition was analyzed, and laboratory studies were performed regularly on all study subjects.

The Nurses' Health Study II was started in 1989 from the same funding source and also sought to study contraceptives, diet and psychosocial risk factors as well as physical activity. Women recruited into this second cohort were younger than those in the earlier cohort, 25–42 years of age.

There is now an ongoing Nurses Health Study III study which was started in 2010. It is entirely web-based, includes men as well as women, different types of healthcare workers, and focuses on diet, lifestyle, occupational resources, and other factors on health.

The Women's Health Initiative

This study was begun in 1991 and was also funded by the National Institutes of Health [24]. Its focus was on postmenopausal women and cardiovascular disease, cancer, and osteoporosis. More than 150,000 women between the age of 50 and 79 years were registered over 15 years. The study consisted of one observational study and three clinical trials. The intervention trials evaluated different types of hormone replacement including estrogen plus progestin versus estrogen alone, dietary change, and calcium and vitamin D supplementation. The major findings were that hormone therapy increased the risk of stroke and did not change risk of coronary artery disease and that the estrogen-progestin combination increased the risk of breast cancer. Dietary modification showed that it lowered the risk of breast cancer in women who had a higher percentage of energy coming from fat only, but not in others. Calcium and vitamins were shown to improve bone mineral density slightly and had no effect on the risk of colorectal cancer as had been hypothesized.

The MacArthur Studies on Successful Aging

Rowe and Kahn were among the first investigators to add a psychosocial component to the definition of successful aging [25]. They assembled a group of 16 investigators from different subspecialties and vantage points and together to initiate the MacArthur Network on Successful Aging in 1984. The group concentrated on studies related to age trajectories of people entering and moving through old age. They confirmed the role of physical activity and social interaction as essential for successful aging, similar to findings from other studies. They also showed that vitality and resilience offset negative factors. They brought successful aging to the forefront of research on aging, showing that aging could be associated with positive growth.

The Berlin Aging Study

The Berlin Aging Study was one of the first major combined cross-sectional and longitudinal studies on aging using randomly selected people living in Berlin, Germany [26]. More than 500 people were selected and were studied in a holistic, multidisciplinary fashion using specialists from internal medicine, psychology, psychiatry, and others to explore physical, mental, cognitive, psychosocial-economic determinants of the population over time. The population was divided into six distinct age subgroups, each one corresponding to a 5-year increment. The study is ongoing, and two cohorts have been followed: the first cohort was selected in 1989 and the second in 1993 and each has been re-evaluated at least several times.

Major findings have been low rates of dementia before age 95 (<10%) but high rates after 95 years of age (greater than 50%). There were relative similarities in these data regardless of wealth for those people who survived into the high age groups, although poorer people were less likely to survive. Subjects had relatively good health over time with 90% of the cohort over 75 years of age reporting successful aging (i.e., cognitively aware and leading an active life) or at least average aging (i.e., relatively healthy, independent, and satisfied with life). Even at 95 years of age, 30% of the people studied still had good or average health. A notable finding was that with age, subjects developed increasing spirituality and serenity. The most important predictors for successful aging were a high level of education and an extended family network.

The Nun Study

The Nun Study was focused on cognitive health in Roman Catholic nuns, all living similar lifestyles [27]. The study focused on cognitive impairment with aging, with a focus on Alzheimer's disease. It was started in 1986 under

the direction of David Snowdon. It was originally housed at the University of Minnesota and followed him to the University of Kentucky when he transferred universities and has since moved back to Minnesota after he retired.

A total of 678 American Roman Catholic nuns constituted the study subjects and were homogeneous in that they drank very little alcohol, they lived together, had similar lifestyles, similar backgrounds, and had no children, husbands, or family responsibilities. They were compliant with study protocols which included regular histories and physical examinations as well as laboratory studies. The study also included review and analyses of writing exercises and autopsy of their brains after death. The studies gave rise to numerous publications about Alzheimer's disease including the possibility of remaining cognitively well despite pathologic lesions given sufficient stimulation and sufficient educational attainment. Those patients who did better were those who had altered circulation in the affected areas of the brain, a component of plasticity in which the brain can remodel itself to heal and limit damage.

The SAGE Study

Jeste and his colleagues, pioneers in positive psychiatry, direct the **S**uccessful **AG**ing **E**valuation study (SAGE) in which 1300 randomly selected people from San Diego ranging in age from 50 to 99 years are under evaluation for types of aging [28]. The subjects are followed with phone interviews, questionnaires, saliva samples, and in-person examinations on a regular basis. In this study, the investigators have found that even as the physical component of quality of life, as measured by the SF-36 survey, declines after 70 years of age, the mental health component appears to improve. Similarly, even as the percentage of people with no disability decreased after 60 years of age, successful aging improved after this time as measured by self-reports.

The Centenarian Studies

Centenarian studies on people over 100 years of age are ongoing throughout the U.S. and around the world including Hungary, Italy, France, Finland, Denmark, and China. Centenarians are the fastest-growing group of people in the world, and these studies may hold the key to long life. The New England Centenarian Study, started in 1995, is the largest study of centenarians in the world and invited subjects who had already reached the age of 103 years, or 100 years if they have siblings who were willing to participate in the study [29]. The study has resulted in more than 140 published peer-reviewed papers. The studies so far have shown that longevity is related to family history, and decline does not necessarily start until the 1990s. Many of the subjects have genes which slow aging and reduce the risk for disease ordinarily related to old age like cancer, heart disease, and Alzheimer's disease. They found 281 genetic markers which could predict living until about 100 years of age.

The Health and Retirement Study (HRS)

The Health and Retirement Study (HRS), which began in 1992, is a large-scale longitudinal project that examines the health, economic, and social factors influencing the aging process. Funded by the National Institute on Aging and conducted by the University of Michigan, the HRS tracks more than 20,000 Americans over the age of 50. The study aims to provide data on the life circumstances of older adults, particularly regarding their health, wealth, and family dynamics. The 2020 wave of the HRS has focused on understanding the effects of cognitive decline and economic disparities on the aging population. The study's findings reveal that economic inequality significantly impacts health outcomes, with wealthier individuals generally experiencing better health and longer life expectancy.

The Harvard Study of Adult Development

The Harvard Study of Adult Development is one of the most comprehensive and longest-running longitudinal studies in history, having started in 1938. Initially, the study focused on two distinct cohorts: 268 Harvard College sophomores, predominantly from well-to-do families, and 456 boys from Boston's inner city, often from disadvantaged backgrounds. The primary aim was to identify the psychosocial predictors of healthy aging, including what factors contribute to a happy and healthy life.

Over the decades, the study has expanded to include the spouses and offspring of the original participants, thereby providing multigenerational insights. The research has explored a wide array of topics, from physical and mental health to marital satisfaction, career success, and the role of social relationships. One of the most significant findings from the study is that strong relationships are the strongest predictor of happiness and well-being throughout life. Loneliness and lack of social connections, on the other hand, have been associated with earlier declines in both mental and physical health.

The study has underscored the importance of not just physical health but also emotional and social well-being in determining the quality of aging. These findings have been influential in shifting the focus of aging research toward a more holistic understanding of what it means to age well.

Other Longitudinal Aging Studies

Table 7.1 lists other longitudinal studies including the Duke Longitudinal Studies of Normal Aging [30], the Kansas City Study of Adult Life [31], the Manitoba Elderly Study [32], the Honolulu Asia Aging Study [33], the Predictors of Successful Aging Study [34], the Canadian Study of Health and Aging [35], the Aging Successfully Until Death Study [36], the Cardiovascular Health Study [37], the Successful Aging over a Lifetime Study [38], the Alameda County Study [39], the NuAGE Study [40], the Successful Aging After Elective Surgery Study [41], the VA Normative Aging Study [42], the Uppsala Longitudinal Study [43], the Helsinki Business Study [44], the National Health and Resilience Veterans Study [45], the Amsterdam Longitudinal Aging Study [46], the Vanderbilt Memory and Aging Project [47], the ACHIEVE-P Study [48], and the Occupational Determinants of Successful Aging in Older Physicians Study [49].

Successful Aging in Adults with Psychiatric Disease

People with psychiatric disorders have shorter lifespans and more difficulty in successful aging than their counterparts. People with untreated depression have a life expectancy of 7–10 years less than the general population [50] and people with schizophrenia have a life expectancy which is 10–30 years less than that of unaffected age-matched people [51].

Major depression and schizophrenia interfere with quality of life by virtue of lower social involvement, greater physical impairment, and decreased life satisfaction, the very factors that decrease the chance for successful aging.

Positive psychiatrists are needed not only to treat patients with these diseases early and aggressively, but to continue treatment beyond return to baseline. Returning them to higher than baseline levels can provide a psychological buffer by enhancing life satisfaction, quality of life, and increasing chances for successful aging [52].

The highest risk period for depression in the elderly is at the time of life events such as loss of family members, disability, and impairment of mobility. Such events can cause or deepen depression and can result in cycles of despair unless treated quickly and effectively.

Jeste et al. have studied aging in more than 1000 subjects with schizophrenia and have shown that their health and survival improved with adequate treatment. With age, as their physical health declined, their quality of life by self-report improved, as did their adherence to treatment,

number of psychotic symptoms, and number of relapses [53].

Of these people with schizophrenia, 145 were older and approximately 10% of them remained in long-term remission. Whether subjects remained in remission related mostly to the amount of social support they received, whether they were married or not, their level of cognitive reserve, and early initiation of treatment [54]. The treatments that were most effective were training in cognitive behavioral social skills, adaptation, and cognitive remediation. Given these data, it behooves the internist and geriatrician to refer their patients with schizophrenia to psychiatrists as quickly as possible to enable them to have a chance at healthy and successful aging.

Not only do people with severe mental illness often not get the psychiatric care they need, they often do not get the primary care they deserve and are often excluded from clinical trials where they could be offered early access to new effective treatments [55].

Another obstacle to effective and quick therapy is the shortage of psychiatrists nationwide and globally [56]. This shortage is acute in the aging population, where attention to mental health may appear to be a secondary goal. Messias' group has suggested that more medical students could be recruited into general psychiatry through focusing on those whose lifestyle and other characteristics best match those of others already entering the field [57]. They also suggested that students be exposed to geriatric and geriatric psychiatry clerkship rotations since such students are more likely than others to express interest in eventual careers in geriatrics or geriatric psychiatry [58]. Alternatively, psychiatry extenders like nurse practitioners and others can help to provide care especially in rural areas. Finally, psychiatrists may also need to train front-line internists, geriatricians, and others to be able to quickly recognize and treat the most commonly encountered mental health issues, or at least refer for specialized services as soon as possible.

Interventions for Successful Aging

Diet

There is only one proven effective method for improving survival and that is dietary restriction. Animal studies have shown that life expectancy can be extended by approximately 50% or more by calorie restriction [60], and certain early human studies have shown benefit of calorie restriction. Witte et al. studied 50 healthy elderly people and subjected them either to a 30% restriction in calories, or a diet enriched in unsaturated fatty acids, or no change. Those people who underwent the 30% restriction had improvement in memory by 20%, whereas the people in the other groups had no change in cognitive function [61]. In a comprehensive review of the literature, Manchishi and others similarly showed that calorie restriction could improve symptoms and signs of depression [62]. Major benefits may have been due to increased insulin sensitivity, less metabolic syndrome, and less inflammation.

There is much controversy about the optimal diet to promote long and healthy life. The Mediterranean diet is replete with vegetables and fruits, a high percentage of unsaturated fat, and low carbohydrates; this diet may be associated with less depression and less risk for cognitive decline compared to other diets [63]. Petersson et al. evaluated the literature on Mediterranean diets and cognitive function [64]. They evaluated 32 studies of which 5 were randomized controlled trials, 25 were cohort studies, and 27 were observational only. People who followed the Mediterranean diet had better cognitive function, less risk for Alzheimer's disease, and less cognitive decline over time compared to their peers. Despite this association however, it is not clear that the diet *caused* the symptomatic improvements. What may be more important than the actual components of the diet may be the reduced calories, relaxed mood, good company, and slow digestion associated with a good meal.

Physical Activity

Physical activity has been shown to be beneficial for cardiac and for brain health. The Framingham studies clearly established a benefit to regular physical exercise at least three times per week [65]. Later, other studies have shown that what is good for the heart is good for the brain [66], and that physical exercise is associated with improved cognitive function, especially as it pertains to verbal memory, attention, cognitive function, and attention [67].

Several pilot studies have provided "proof of principle" that exercise can improve cognitive function. In one large study, elderly subjects over 50 years of age with mild cognitive impairment as manifested by memory problems were randomized to exercise intervention compared to non-intervention [68]. Cognitive function improved significantly in those people doing the exercise program compared to the others.

Physical exercise has been studied in subjects with mental health disorders, notably anxiety, depression, and schizophrenia. Wegner et al. reviewed meta-analyses exercise effects on anxiety and depression. They identified 37 meta-analyses, encompassing more than 40,000 subjects. They found that exercise improved symptoms in both conditions, but more so in subjects with depression than those with anxiety [69]. Josefsson and his colleagues reviewed the literature and data on the effects of exercise in depression [70]. They found 13 high-quality studies and concluded that exercise improved symptoms, especially in the highly controlled studies and especially in subjects with mild to moderate depression. However, the quality of the studies was very variable.

Similarly, Firth et al. studied existing meta-analyses of studies about exercise and cognitive outcome in people with schizophrenia [71]. In all, they reviewed ten studies covering 385 subjects. They found that exercise improved global cognitive function, and it was directly related to the amount of exercise. Also, exercise was more effective when done in a supervised manner and worked especially well on memory, social cognition, attention, and vigilance.

Erickson and others showed that exercise can increase the size of the hippocampus and improve memory [72]. Ordinarily, the hippocampus shrinks in late adulthood and this decreases memory and increases risk for dementia. Exercise increased hippocampal volume by 2%, reversing approximately 2 years of age-related deterioration. The increase in hippocampal size was associated with increases in circulating brain-derived neurotrophic factor (BDNF) which mediates neuronal growth and recovery.

Cognitive Stimulation

Castel and his investigators evaluated 176 people randomized to one of two groups to either receive cognitive stimulation or not [73]. Those who partook in the cognitive stimulation program had improvement in psychological well-being whereas the others did not. In support of this, Kelly and his colleagues reviewed all the literature and did meta-analysis of then existing cognitive training and stimulatory programs [74]. In all they evaluated 31 randomized, controlled trials encompassing more than 1800 people in training compared to more than 350 controls. Overall, people receiving cognitive stimulation had improvements in working memory, processing speed, composite cognitive function, and face and name recall.

It seems that cognitive stimulation can be incorporated into a leisure program rather than a formal training session, which may make the training more palatable to some. In this regard, Grimaud et al. showed that subjects doing cognitive stimulation work either in a standard training program, or integrated into their leisure activities did equally well in improving speed of processing, working memory, and self-esteem [75].

These programs may work best in subjects with mild cognitive impairment and less well with marked impairment. In studies of people with clinically significant dementia, Bailey and others showed little improvement with stimulation. They randomized subjects to ten sessions each of cognitive stimulation or none. There were no differences in outcome in those randomized to

either ten sessions of cognitive stimulation or no stimulation. This suggests a "point of no return" after which patients will no longer respond to stimulatory activity.

Combining Physical Activity, Cognitive Stimulation, and Fun Through Exergaming

Exergaming is a way of exercising with an internet-based game platform such as Nintendo to combine sensory, motor, and cognitive input, thereby increasing physical tone and motor skills [76, 77]. Exergaming using these platforms with TV and sports equipment can be done to play virtual golf, tennis, soccer, football, or other games. It builds stamina in a fun and progressive manner, is inexpensive, easily accessible, and easy to set up in one's own home. Data is not available to compare different types of exergaming to each other or to actual gaming.

Social Participation and Engagement

Loneliness is one of the major risk factors for adverse physical functioning and one of the major impediments to successful aging. Lee et al. evaluated loneliness in 340 elders living in San Diego, CA. Subjects were 27–101 years of age (mean age 62) [78]. Of these residents, 76% had moderate to high amounts of loneliness and this was related to worse mental health. Only three factors influenced this problem: wisdom, living alone, and mental well-being.

Loneliness is therefore a key obstacle to successful aging, and also a key point at which to intervene. There are many ways to combat loneliness. Any social meetup, time with others, volunteering, or involvement in synagogue, church, or mosque activities helps to break down isolation. Similarly, caring for a pet or even for a plant is beneficial for connecting with the outside world. Whether interaction with robots, or artificial robotized animals and people would similarly be effective is not known; however, there is a huge developing industry for creating such toys with the hope that they will compensate for loneliness in the elderly [79].

A new and very promising concept is that of the age-friendly community (AFC) initiative [80, 81]. In these communities, elders live together in communities and age-in-place together as their physical function declines. The most prominent of these types of living communities are within the World Health Organization's Global Age Friendly Communities (AFC) network [80]. This network has 287 communities in 33 countries. A similar model is that of the AARP network of AFCs with more than 50 communities throughout the U.S. [81] Each of these communities has affordable housing, safe green space, senior-friendly environments, easy and inexpensive transportation options, activities, and health and wellness services.

Some of these homes for the elderly are armed with sensors which can indicate when elders are in need, when they fall, get lost or develop medical problems [82–84]. These sensors can then mobilize an internet-controlled device such as a robot to intervene, help, call for help, give medication, provide support, or alert Alexa, Siri, or other internet messenger to call someone. This machine-to-machine interaction to control the outside environment and make it safer constitutes an internet-of-things platform.

This type of platform can also be used to connect one's environment with a hospital setting through augmented intelligence digital health systems. In this set-up [82], biomedical and psychosocial information can be sensed, transmitted to the health center, interpreted, and response can be returned through internet-controlled communication or robot systems.

Reinforcing Positive Psychological Traits

The main psychological traits associated with successful aging are optimism, resilience, and wisdom. Optimism is associated with better quality of life, better survival, and more successful aging trajectories. In one study, optimism in elderly men was shown to remain stable over

time and was inversely and directly related to loneliness and risk of cardiovascular disease [85]. Resilience is also associated with less depression and better aging [18]. In the Successful AGing Evaluation (SAGE) study, Jeste et al. found that higher scores on a self-reported measure of resilience were associated with low levels of depression [18].

Wisdom seems to be a quality that ripens with time and is also associated with less depression [19, 86–88]. There is no simple definition for wisdom, rather it has many components and varies by culture. Qualities of wisdom include (but are not limited to) emotional regulation, altruism, empathy, reflection, tolerance, and insight [87].

Despite the disparate components of wisdom, its site can be mapped on the brain [17]. Jeste et al. have shown that this function resides in the dorsolateral prefrontal cortex, the anterior cingular cortex, the ventromedial prefrontal cortex, and the amygdala [17]. Knowing the location in the brain and being able to find it may help in monitoring the growth and development of wisdom over time, as well as before and after interventions.

Jeste and his colleagues have suggested that wisdom is a particularly important personality characteristic that is fostered with age and have even suggested that the purpose of long life after reproductive capability may indeed be to develop—and share—this quality. Wisdom offsets the negative impact of physical and mental decline inherent in aging, death and dying and offers benefit to one's community, society, and world. Neller and her colleagues found that when planning for the end of life, older adults viewed values and life lessons as the most important things to pass on to loved ones. A world with wisdom would be less prone to hatred, violence, and war and would be a better place to live.

Of course, the major challenge would be to channel wisdom generated from the aging population into the younger population at large. The transmission of wisdom from older to younger generations may also be viewed as legacy, which Neller et al. [106] defined as "what one leaves behind after death, making a lasting impression, leaving a mark on the world, personally contributing to the future, or leaving a portion of oneself." Difficult as this appears to be, it may actually be possible to pair young people with old people in social networks such as school systems. Thus, the University of Arizona has an "optimal aging program" in which students are paired with older adults as their mentors [89]. Students thus hear about challenges of aging as well as lessons of a lifetime while seniors feel useful, connected, and valued.

A similar project is "The Experience Corps," sponsored by the American Association of Retired Persons (AARP) [90]. This program teaches seniors over 50 years of age how to tutor children and then pairs their volunteers with children throughout the U.S., mostly in poor inner-city neighborhoods. Seniors then work with these children on a one-to-one basis or in small groups. To date, more than 300 senior volunteers have been trained. These volunteers have helped more than 4500 children; conversely the children have helped them by giving them a sense of altruism and usefulness.

Another more recent project is the "Nones and Nuns Project" in which young millennials who have no religious affiliations (i.e., religion affiliation = "none") have paired with Catholic nuns [91]. Together they meet at Catholic churches and monasteries around the country to dialogue and for "nones" to learn wisdom and spiritual practice from nuns. Nuns benefit too, from sharing their wisdom and from feeling useful and valued.

Meditation

Meditation may have a role to play in successful aging but has not yet been well defined.

Sperduti compared elders with long histories of meditation and compared them to age-matched people with no experience in meditation [92, 93]. In this cross-sectional study, the non-meditators had significantly lower executive function, especially memory and attention, compared to their peer group who were experts in meditation.

This study concords with work by Luders et al. comparing brain age of people who meditate compared to those who do not. In this study, the investigators studied 50 elders with long histories of

meditation compared to 52 age-matched subjects without a history of doing meditation [94]. At 50 years of age, brain age of the meditators was approximately 7.5 years less than that of the non-meditators, using BRAINAGE index, a radiographic procedure validated to measure the age of the brain [95]. Similarly, Chetelat et al. evaluated six long-term meditators compared to 50 control subjects and found that the meditators had higher amounts of gray matter in their brains and higher glucose metabolism compared to the non-meditators [96].

It therefore appears possible that meditation may be able to slow aging of the brain. In order to study this more directly, some investigators have studied the effects of meditation on telomere length before and after mindfulness meditation as well as before and after stress. Epel has recently reviewed his and other studies to show telomere shortening with stress and promotion of telomere stability with meditation, evidence of slowing of the aging process with meditation [97].

Some investigators have evaluated different forms of meditation on telomeres, a marker of overall biological age and health. For example, Nguyen and colleagues studied mindfulness meditation compared to loving-kindness meditation on telomere length, each compared to controls of people who do not meditate [98]. In their pilot study of 12 weeks of mindfulness versus loving-kindness versus no meditation, only the people doing loving-kindness meditation preserved telomere length, while the others had telomere shortening over time.

These studies suggest a possible benefit to meditation on aging. More studies are needed to determine optimal type and timing of meditation and a plethora of related other issues.

Medication: New Psychiatric Treatments

Positive psychiatrists also have psychiatric drugs to use in their pursuit of bringing meaning and joy to their patients with, or at risk for developing, mental health issues and others. In particular, psychedelic drugs are now being rediscovered [99, 100]. Thus, psilocybin (from "magic mushrooms"), mescaline (hallucinogen from cactus plants), MDMA (3,4-methylenedioxymethamphetamine, or "Ecstasy") are now being evaluated for mental health disease and to augment response of the brain to behavioral and other psychotherapies. Ketamine and esketamine are other drugs which have now been approved for patients with severe depression refractory to other treatments [101]. Such therapies could help positive psychiatrists to reprogram their patients with healthy, positive messages to promote successful aging.

Psilocybin, a naturally occurring psychedelic compound, has recently gained attention for its potential to treat depression and anxiety in older adults. Studies indicate that psilocybin may enhance neuroplasticity and reduce symptoms of depression, offering a promising new avenue for improving emotional well-being in aging populations [107].

Ongoing research into stem cell regeneration of the brain, brain-derived neurotrophic growth factors, and superficial and deep brain stimulation also hold promise too for better treatment of patients with mental health issues [102]. There are also dozens of new drugs which are advertised to help slow and/or reverse the aging process, many of these however are currently unproved therapies and should be used only in clinical trials.

For people suffering from Alzheimer's disease, new monoclonal antibody infusions targeting amyloid have recently been approved by the FDA in the United States. Amyloid is a misfolded protein that builds up in the brain of people suffering from Alzheimer's disease, and is considered a major contributor to cognitive decline in this illness [108–110]. These treatments help clear amyloid from the brain, and these treatments are hypothesized to slow and possibly even stop the progression of cognitive decline in Alzheimer's disease. Time will tell how effective these treatments actually are.

Others

Other interventions, nutritionals, pharmaceuticals, psychotropics, and even psychedelics have been advocated for successful aging and are listed in Table 7.2. Many of these therapies however have not been adequately tested and many are being developed as quick snake oil type products.

Table 7.2 Interventions to foster successful aging

Interventions	Outcomes
Calorie Restriction	Improves blood pressure and lipid profile
	Increases median and maximal longevity
	Improves memory
	Increases metabolic efficiency and insulin sensitivity
Exercise	Reduces anxiety, depression, and risk for dementia
	Improves executive function
	Reduces functional decline, preserves physical functioning, may prevent disability
Cognitive stimulation	Brain training games improve cognitive function
	Games can improve memory, attention, driving skills, balance, among other skills
	Can be combined with smart technology (i.e., exergaming)
	Exergames increase physical activity, fine and gross motor skills
	Widely available, easy to set-up, inexpensive, can be done at home
	Exergames include tennis, golf, rowing, shooting, stretching, yoga, cycling, and many more
Social engagement	Loneliness is a major factor for unsuccessful aging
	Social engagement mitigates depression, cognitive impairment
	Age-friendly communities (AFC) provide resources to engage socially
	World Health Organization maintains AFC network
	AARP maintains network
	Many programs (i.e., experience corps, Nones, and nuns project) engage elders in the community which may be sponsored by universities, AARP, and cities
Promoting personal traits	Optimism, resilience, and wisdom mitigate against loneliness, depression, and despair
Meditation	Unproven, but interesting initial results
	Mindfulness meditation may improve relaxation, perceived control
	Transcendental meditation may improve cognitive and behavioral flexibility and learning
Medication	Psychedelic drugs are being rediscovered (psilocybin, mescaline, MDMA)
	Stem cell infusions for regenerative brain structure; brain-derived growth factor
	Anti-aging industry has many new products but most are unproven
	Brain stimulation with transcranial direct current may stimulate selected areas of brain and may be useful when paired with psychiatric care
	New monoclonal antibodies have been approved to slow/stop the progression of Alzheimer's disease
	Hormones (testosterone, estrogen, growth hormone, oxytocin, etc.): Unproven
On the horizon	Smart homes equipped with real-time monitoring of vital signs, EKG, mobility
	Internet of things can connect sensor with internet assistant (e.g., Siri, Alexa), robot, or hospital
	AI-enabled tools are emerging to offer personalized, predictive analytics, enabling early detection and tailored management of age-related health conditions
	Advances in wearable technology allow continuous monitoring of vital signs and activity, empowering older adults and enhancing personalized healthcare
	AI-enabled digital companions are being developed to boost social engagement and cognitive health in older adults by providing conversation, stimulation, and activity reminders

Implications for Positive Psychiatry

Successful aging from the bio-cognitive-behavioral psycho-socio-economic standpoint is not genetic but is most rather mostly related to attitudes, beliefs, behaviors, and lifestyle [103, 104]. The most important psychological traits for successful aging are optimism, resilience, social engagement, and wisdom [104]. Harnessing these traits can overcome much suffering. Successful aging can thereby be possible, even in patients with serious mental health illness. However, underlying mental health conditions need to be treated early and aggressively for patients to age successfully like their counterparts.

Successful aging is a complex process in which biomedical, cognitive, behavioral, and psychosocial factors are all important and are all interconnected. With the aging population, more resources will be necessary to care for elders and to help them derive as much satisfaction from life as possible. Positive psychiatry has much to offer in reinforcing positive traits to cope with the challenges of aging, and to restore function through diet, exercise, cognitive stimulation, meditation, and medication.

Positive psychiatrists are especially needed to help people with mental health disorders or at risk for developing them in overcoming the negative impact of their disease and for improving their lifespans and health spans. The goal must be to not only bring patients with mental health issues back to their baseline, but to a much higher level so as to provide a buffer and to offer enhanced meaning and quality of life [52].

Most of the underlying reasons for actual cause of death in these days arise from psychosocial issues including smoking, obesity, poor nutrition, physical inactivity, opioid and other addictions, suicide, and homicide [59]. These issues require prevention, early detection, treatment, rehabilitation, and maintenance programs. Positive psychiatrists can develop such programs and help restore health and meaning to affected people, either as culprits or as victims.

Perhaps most interesting for the positive psychiatrist is the possibility of enhancing and spreading the positive aspects of aging. With aging comes gifts of experience, kindness, wisdom, serenity, and others [103]. If these gifts can be enriched and disseminated, they would become treasures not only for seniors themselves but for everyone, making the world a kinder, gentler, happier, and safer place.

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8 Life Balance with Positive Psychotherapy

Hamid Peseschkian and Arno Remmers

> Many people seem to think that success in one area can compensate for failure in other areas. But can it really?... True effectiveness requires balance. –Stephen R. Covey [1]

Introduction

Inspired by a concept in the Bahá'í Writings about "The Four Criteria of Comprehension," Nossrat Peseschkian developed the balance model as an integral part of his psychotherapeutic method [2]. Here, the influence of the positive view of man is seen to be renewed, a concept of a healthy person is presented and the ways through which such health can be attained are shown. This means that a health-oriented conceptualization can proceed from concepts of salutogenesis rather than pathogenesis. It was important for Nossrat Peseschkian to tell patients not only what they had done wrong, but above all to offer them a concept of life for their own orientation. Peseschkian's concept of the balance model [3] is known as the very heart of positive psychotherapy (PPT after Peseschkian) and is used in different settings. It is also known as the "rhombus model," the "four qualities of life," the "four ways or medians of the capacity of recognition," the "four areas of life," the "four ways of conflict resolution," or the "diamond model" [4] (see Fig. 8.1).

The balance model is based on the concept that there are essentially four areas of life in which a human being lives and functions. These areas influence one's satisfaction with life, one's feelings of self-worth, and the way in which one deals with conflicts and challenges. They are the hallmarks of one's personality in the here and now. This model describes and connects the biological–physical, rational–intellectual, socio-emotional, and imaginative–spiritual spheres and capacities of human beings in everyday life. Although the potential for all four areas and capacities can exist in every human being, some are emphasized and others neglected through differences in culture, family, education, time, and environment. The four areas of life, energy, activities, and reactions are:

Physical activities and perceptions, such as eating, drinking, tenderness, sexuality, sleep, relaxation, sports, appearance, and clothing

Professional achievement and activities, such as a job, work, studying, household duties, gardening, basic and advanced education, and money management

Relationship styles and contact with partner, family, friends, colleagues, acquaintances, and strangers; social engagement

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_8

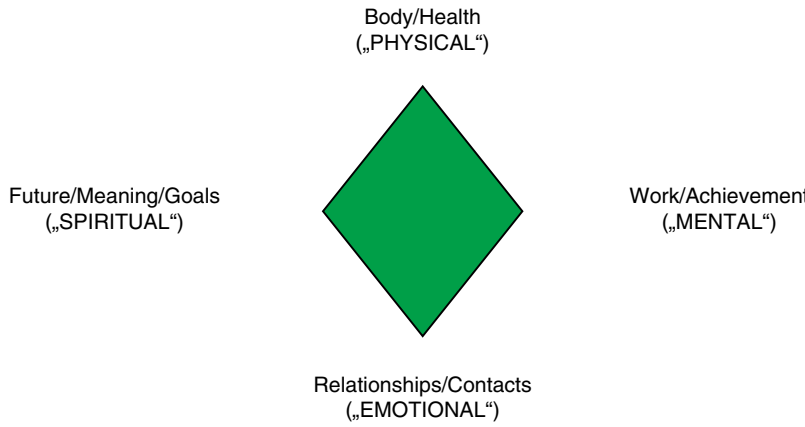


Fig. 8.1 The balance model of positive psychotherapy

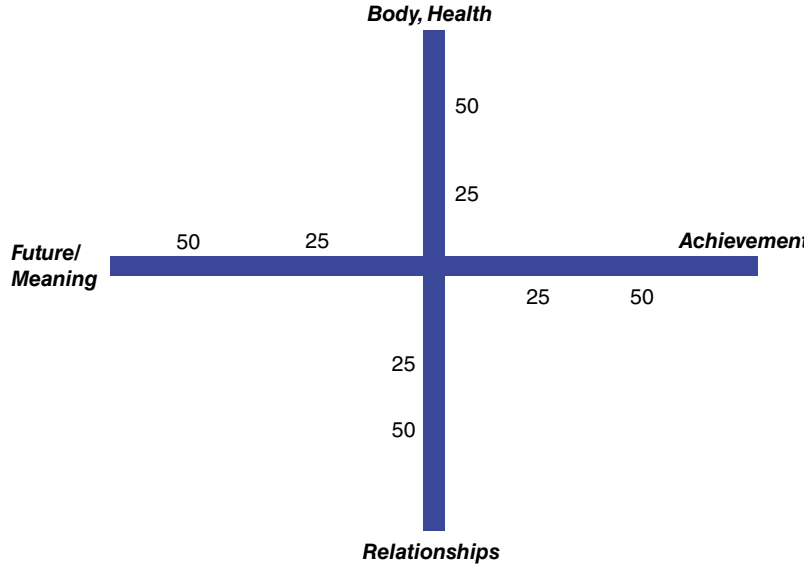


Fig. 8.2 Energy distribution in the balance model

Future plans include religious/spiritual practices, purpose/meaning, life goals, meditation, self-reflection, death, belief, and the development of vision or imagination–fantasy

Nossrat Peseschkian sees a balance of the energy of life distributed among the four areas as favorable conditions for health and resilience: *"According to the concept of Positive Psychotherapy, a healthy person is not one who has no conflicts, but rather one who has learned to deal adequately with these conflicts. Here "adequately" means not to neglect any of the four areas of life, but to distribute one's energy (not necessarily time!) approximately evenly into the four areas of life."* [5], p. 99. The goal is to achieve balance in the four areas. Accordingly, one objective of psychotherapeutic engagement is to help the patient to recognize his own resources and mobilize them with the goal of bringing them into a dynamic equilibrium. This places particular value on a balanced distribution of energy (approximately 25% to each area), not an equal distribution of time (see Fig. 8.2).

One-sidedness that goes on far too long can lead to conflicts and disorders, among other things. *"The four areas call to mind a rider who, being motivated toward achievement (Achievement), strives to reach a goal (Meaning/Future). For this he needs a good horse which is well-groomed (Body), and just in case the horse should throw him off, helpers who will assist him*

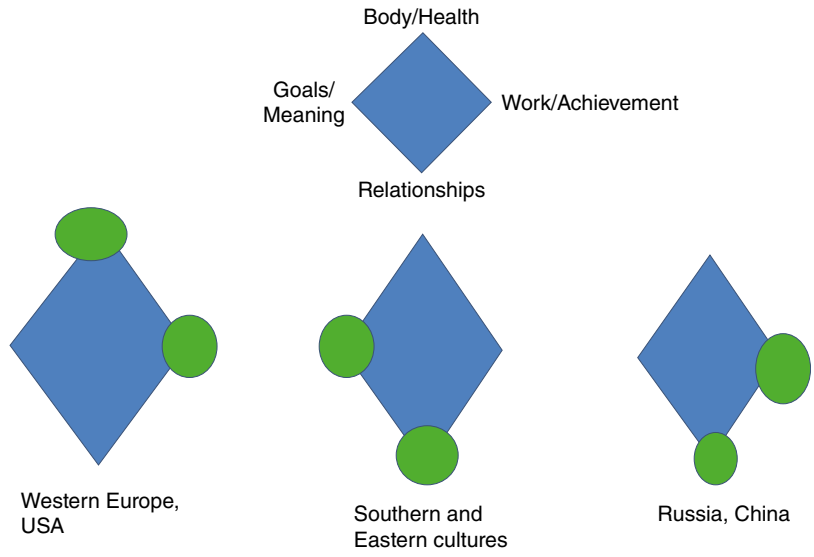


Fig. 8.3 Balance model in different cultures [7]

to remount (Relationships). This means that therapy cannot deal only with one area, for example, the rider, but must consider all related areas" [3].

Consideration of the uniqueness of the patient is important so that he/she can reach an equilibrium within the framework of the "four areas," that are agreeable to him/her. To put it simply: every person should be in balance by not neglecting any area of life for a long time; however, the manner of this balance is specific to individuals, families, and cultures. For example, more individualistic cultures such as in Western Europe and North America value the areas of body/health and achievement/job more importantly, whereas in more collectivistic cultures such as in Southern Europe, the Middle East, many parts of Asia and Africa, the area of relationships (family, friends, relatives) and questions about the future, the meaning of life, and about one's worldview (area of future/meaning) are more significant to them [6] (see Fig. 8.3).

From Work–Life Balance to Life Balance

The positive conception of human beings becomes particularly obvious when seen in the interpretation of the individual life balance. Instead of pointing out a deficit to the patient or his/her family, and giving some immediate advice, the positive aspects of one-sidedness are emphasized and encouraged. His/her weakest feeling of self-worth is strengthened to create a basis for the analysis of the areas with deficits. For example, a person who places a high value on achievement and works long hours every day is not confronted at an early stage with the fact that he or she should spend more time with the family (see Fig. 8.4). At the outset, his inclination for achievement and his motivation to work are seen and identified as a capacity. This is an encouraging experience for the patient and is important and fruitful for building the relationship between the therapist and the patient. At the same time, the patient and his family are told that the patient is not suffering because he/she works so much, but rather that he/she has neglected other areas of life and not developed them. Often patients and clients are very surprised and say, "You are the first one who tells me that working so much is a capability and not only negative."

In this context, a positive image of human beings means a resource-oriented process, which means that this patient has the capacity for balance, but, owing to his/her socialization and the circumstances of his/her life, he/she has not developed certain areas of life until now. This process also prevents the phenomenon that in

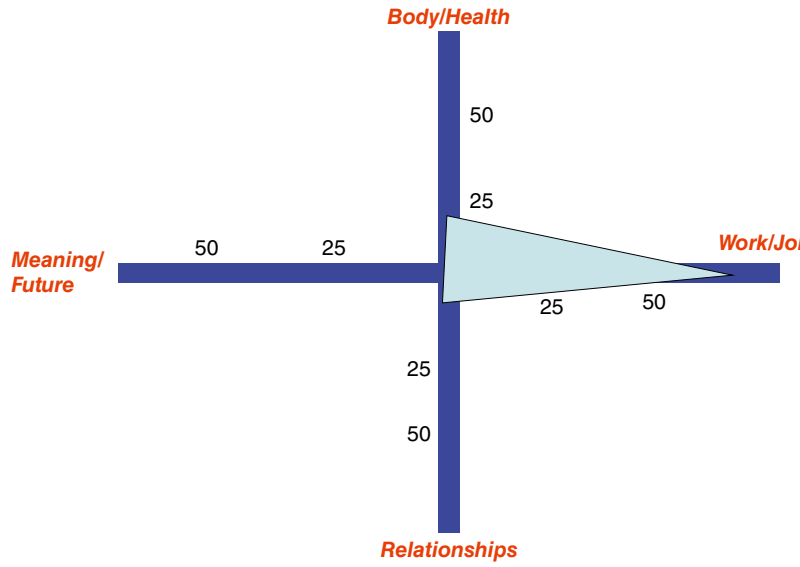


Fig. 8.4 Life balance of a work-oriented person

psychiatry is called relief depression. Thanks to the therapy, he/she reduces his/her workload, goes on forced leave with his/her partner and children, paces back and forth the whole day at the beach, and spends the whole day on the cellular phone or the computer until his/her partner cannot stand it anymore and sends him/her home early.

Areas in which we have invested a great deal are generally our greatest sources of self-worth and affirmation. When such an area is diminished before another one has been positively filled in, the patient falls into a deep hole. There is too much energy that cannot be invested; thus, it goes into full-blown inner unrest. A feeling of low self-esteem ("nobody needs me") is only one of its consequences, but perhaps the most important.

During the past 30 years, the term "work–life balance" has become part of society's daily conversation and goal in life. Thanks to this concept, people started to give importance to other areas of life. Unfortunately, the term "work–life balance" itself causes stress, as it suggests that there might be work and that there might be life. In many countries, it is neither possible nor desirable to separate between work and other areas of life, owing to technology, home office, different settings, etc. Many patients report that they feel guilty and are under enormous pressure not to think or speak about work at home. Therefore, we use the term "life balance" in positive psychotherapy instead of "work–life balance." The goal is to integrate all spheres of life into one's life. After all, the two are parts of one person. Thus, we need more work–life integration than work–life balance.

In positive psychotherapy, the therapist draws the attention of the patient to his/her strong areas and to those not yet developed. While the patient continues with his/her old lifestyle, the background of his/her one-sidedness is worked on. New areas are being filled with desire, so that they can affirm his/her self-esteem.

The following *questions* may help one to reflect on one's own balance:

1. How much energy do you use for your body, for example, for sports, clothes, relaxation, rest, sexuality? How important to you is this area and how much of your self-esteem is obtained from it?
2. How much energy do you use each day for education, profession, homework, or other capabilities? Could you imagine a life without work, if you were financially secure? How much of your confirmation do you obtain from your profession?
3. How much significance do family, acquaintances, or friends have in your life? How great

is your need for contact? Do your partner or children give you confirmation of your self-worth or do you receive your confirmation from other areas of life?

4. How much space does fantasy take up, for example, reading, music or painting, or just thinking about life, beliefs, your own future, or humanity? Do you occupy yourself much with these questions or only with specific events? Do you think often about the meaning of your own life?

Four Ways of Coping with Conflicts and Conflict Resolution

Despite all cultural and social differences and the uniqueness of each person, we can observe that all people resort to typical forms of working through conflicts to cope with their problems. When we have a problem, we feel angry, burdened, or misunderstood, we feel stressed all the time, or can see no meaning in our lives. We can find expression for these difficulties in the four forms of working with conflicts according to how they are mapped out in the balance model. They let us know how we perceive ourselves and the world around us and also through which method of acquiring knowledge the reality check succeeds.

Each person develops his own preferences consciously, or mostly unconsciously, as to how to deal with conflicts that appear.

> The model of the four qualities of life also presents the basis for the conceptualization of typical conflict models. We flee into illness (somatization) or excessive body-building, into activity and achievement (rationalization in the sense of disturbances of overburdening and adaptation), into refusal to perform, into loneliness or gregariousness (accompanied by idealization or deprecation, which lead to affective disturbances and changes in social behavior) and into fantasy and the world of thought (denial in the sense of anxieties, phobias, panic attacks and disturbances of illusion, addictive behavior, for example, into lack of imagination. [5], pp. 99 ff

Which form of conflict resolution one chooses depends largely on one's experiences, especially those from one's childhood. The balance model is used in such a way in the presentation and differentiation of the four areas of conflict reaction as to describe disturbances of illness and symptoms. In this way, Nossrat Peseschkian differentiates between those areas that are overemphasized and over-differentiated in dealing with conflicts from those that have been pushed into the shadows in an effort to deal with the conflict.

The Balance Model in Practice

The balance model can be applied in a variety of areas, such as psychotherapy, coaching, counselling, mediation, professional assessment, prevention, personal and professional goal setting, management training, stress management, psychotherapy education, and even in choosing a partner and marriage preparation.

Here is an example of how the symptoms, conflict reactions and resources can be documented using the balance model during an interview (see Fig. 8.5):

- The patient's individual goals, available resources, and developed areas can be allocated to the areas of the balance model and discussed with the patient. This also promotes insight into correlations and the motivation to change. Negatively formulated goals, such as "not wanting to have anxiety anymore," are turned into positively formulated realistic goals.
- The balance model is also used by therapists to describe their countertransference: what they think and feel about the patient, specific experiences of interaction with the counterpart, and the therapist's fantasies can be put into the balance model and used to understand the material gathered during the sessions.
- We can record symptoms, conflict reactions, and resources into the balance model during the therapy session. Eventually, the patient will thereby attain an understanding of his/her life situation and of the effects of his/her disorder.

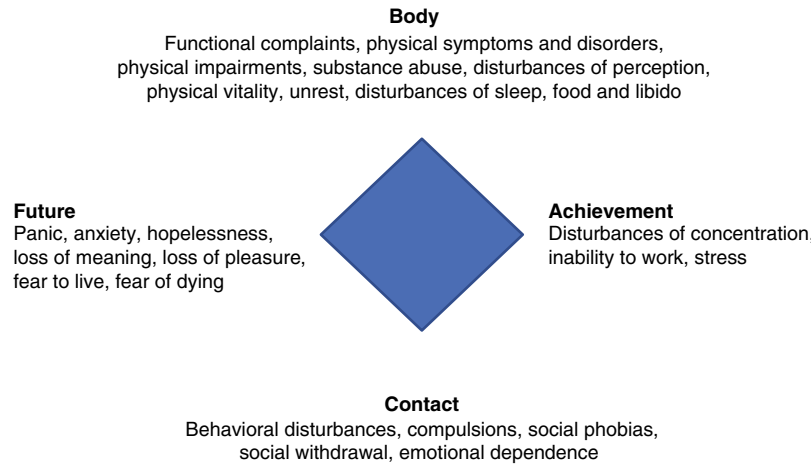


Fig. 8.5 Example of a documentation sheet during the first interview

Case Example and Application

Here is an example of a 45-year-old female patient during the first session (see Fig. 8.6):

- *Body/Health*: The patient's current and previous illnesses and complaints, in addition to her physical resources, are documented. Goal-oriented questions in the first interview concerning nutrition, sports, sexuality, relaxation, and sleep complete the spontaneously provided data.
- *Work/Achievement*: We enter spontaneously given and elicited data about the following aspects—profession and the ability to work, other capabilities, housework, relevant compulsive symptoms, difficulties at the workplace, date when hired or when the job was lost, or financial burdens such as housing construction.
- *Relationships/Contacts*: Here, we find a brief overview of relationships and social resources, which include problems, dealings, friendships, data on the social environment, associations, etc.
- *Future/Meaning*: She has concerns about the philosophy of life, about that which gives support in life, about anxieties, dreams, beliefs, religion, politics, goals, desires and creative activities, the goals in life, and of therapy.

Besides symptoms and disorders, patients suffer a lot with regard to their influence on daily life. In addition to the report of the patient, the following questions can be asked about the effect of the *complaints* and disorder in the four areas:

- What are the effects of your complaints on your physical constitution?
- How have your sleep, sexuality, eating, or pleasure changed during this time?
- How do your complaints affect your ability to work and your present job?
- How does your partner (your family) react to your complaints?
- How have your relationships in the family or with friends changed because of your complaints?
- Who shows an understanding attitude toward your suffering?
- Who acts differently toward you than before?
- Which contacts have been most badly affected by your complaints?
- How do you see your future and that of your family being influenced by your illness?
- What gives you support and what makes you anxious?
- What goals or desires do you have now that you did not have before?

In the case of this patient, the balance model was presented in its function of describing the

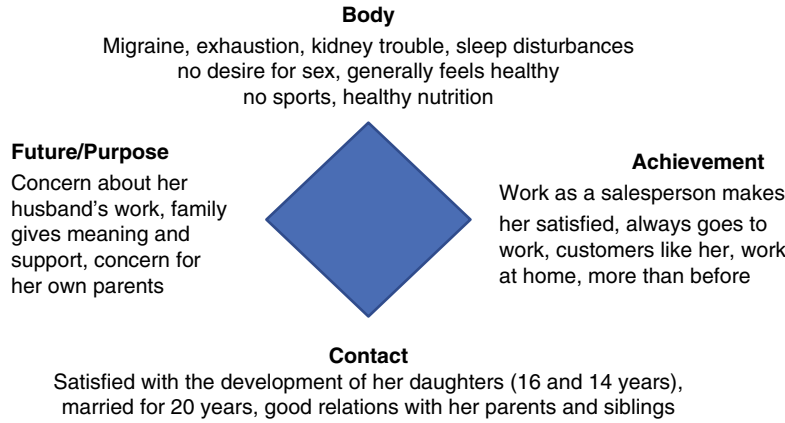


Fig. 8.6 Symptoms of a patient in the four areas of the Balance Model

effects of the symptoms. Through this half-structured approach with questions, unexpected ideas, conflict reactions, experiences, and resources come to the surface in the four areas, particularly in the second stage of therapy. The comparison of the patient's balance model with that of his/her partner provides the means through which to differentiate the social resources, conflict reactions, and conflicted areas.

Questions for the Assessment of Life Balance

There are questions used to effectively assess the balance of the patient. These questions [3, 8] can be given to the patient as homework and reviewed at the next session(s). For example, "How do you see your own balance among these four areas?" with reference to distribution of energy by using Fig. 8.4. Working with the balance model leads to further stages of therapy. Often in these discussions, not only the contents of conflicts but also typical conflict reactions, basic concepts, mottos, and model dimensions become clear.

Questions for the Area of Body/Health

- How do you judge your appearance?
- Do you view your body as friend or foe?
- Which of your organs reacts to anger, anxiety, or fear?
- How do you sleep?
- Are you satisfied with your sex life?
- Do you do some sport? What kind of sport? How intensively?

Questions for the Area of Work/Achievement

- Are you satisfied with your present job and your learned profession?
- What profession or job would you like to practice?
- Which professional activities are giving you a hard time?
- How do you feel when you have nothing to do? Can you do "nothing"?
- Would you still continue working, if you had enough money?
- What role does money play in your life?
- How do you react when a supervisor or a colleague criticizes you?
- Which of your parents put more emphasis on achievement?

Questions for the Area of Relationships/Contact

- How do you feel when you are in a gathering of many people?
- Do you like to invite guests (family, friends, colleagues)?

- How often do you go to the movies, theatre, concerts, or other events? With whom?
- Are you a member of an association? Do you participate actively?
- Are traditions (family, religious, cultural, political) important to you?
- Which of your parents was more sociable and liked contact?
- In your childhood, to whom could you turn when you had a problem?

Questions for the Area of Future/ Meaning

- Do you consider yourself an optimist or a pessimist?
- Do you sometimes think about the future? About your future, that of your family, your country, the world?
- Do you believe in life after death?
- What gives you support in your life?
- What is the purpose of your life?

A Brief Comparison of the Balance Model with Similar Concepts

A comparison of the balance model with the models presented by other authors shows similarities between this anthropological approach and other therapeutic orientations. What Nossrat Peseschkian calls the "energy of life" is approximately equivalent to "libido" or a general energy, as described by Carl Jung or Alfred Adler. Jung also presents as the goal of therapy and the way of life a balance among the four areas of "function types": thinking, feeling, perception, and intuition. When all four of these functions have been brought to the conscious level, the entire crisis can stand in the spotlight; thus, we can speak of a "rounded" person [9], p. 21; [10].

The operationalized psychodynamic diagnosis (OPD-2) includes four observable basic capacities of the personality structure, which are similar to the "four means of the capacity to know" [11], as described by Remmers [12]:

- The "means of the senses" is comparable with the structural capacity to perceive oneself and others, as described in the OPD-2. In the foreground, there stands the body—the self—the feeling.
- The "means of reason" serves in Peseschkian's balance model as the reality check through which problems can be resolved systematically, and it directs our activities. It is related in OPD-2 to the structural capacities that direct our inner and outer impulses.
- The "means of tradition" serves as the capacity to take up relationships and to flee from them. In OPD-2, the analogy to this is the emotional communication with oneself (internal dialogue) and with others as the structural capacity for empathy, and the anticipation of relating to and thinking about others.
- The "means of intuition" is described by Nossrat Peseschkian as the fourth area, that of meaning, future, and fantasy. He defines it as the capacity to imagine something in one's thoughts. It can allow for the sudden appearance of the vision of a painful separation from a partner. Intuition and imagination can go beyond immediate reality and take in whatever we can depict as the meaning of an action, the meaning of life, desires, pictures of the future, or utopia. In the OPD-2, the fourth capacity of the personality structure is the "capacity to form attachments" [13]. Included in this is the imagination of objects that provide support, the connection to an ideal, and the external connections with persons. In Nossrat Peseschkian's scheme, we understand this as the capacity for imagination. With its help, the small child can imagine his/her mother, so that after a short period of being alone, just imagining her can quiet him/her, in contrast to those with structural disturbances, who often find it impossible to summon up such an imagined image. In this sense, the distribution of the four areas of the balance model presents a forerunner of the structural models in OPD-2 (English translation [13]).

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9 Death and Grieving with Positive Psychotherapy

John E. Okoro

Introduction

"Birth and death have often enough been described as entrance and exit. But while a handful of expressions suffice to describe entry into this world, there are hundreds of ways in every language to express its departure, and one can gauge how vividly human imagination has always been concerned with the problem of death" [1]. Despite this, death is a subject that our society, with its worship of youth and orientation toward progress, tends to overlook, ignore, and deny. It is almost as if we see death as merely another disease to overcome. Yet, the fact remains: death is inevitable. We will all one day die—it is simply a matter of timing (ibid.).

From a practical perspective, death is a natural part of life. Every life that begins eventually ends. No person attending a play expects it to go on forever. When you visit someone, you will eventually say goodbye. A movie begins and ends, as does a book, a holiday, or a relationship. Every story has a beginning and an end [1]. Whatever we experience, whether joy or sorrow, war or peace, night or day, success or failure, will always come to an end.

The statement that the question of death and dying has always occupied people is one of the standard introductions to works on the psychology of death [1]. From the Indian Vedas 3,000 years ago to contemporary thinkers, the goal has always been to interpret the meaning of death and help people out of fear and anxiety (ibid.).

We experience death daily through the little farewells of everyday life, which serve as a preparation for the finality of our own lives. The way we handle these farewells varies across cultures. However, it is a common spiritual belief in many cultures that those who have learned to live fully and focus on the present can also accept dying more easily. Drewermann summarizes this belief as follows:

> What are we humans in the face of the omnipotence of death? Or vice versa: what is death if it is always there as a threatening object or, depending on the circumstances, also as a liberating possibility, as a chasm or a loophole, as a grave or mercy, depending on where we stop and start? One thing is certain: only people who are presumptuous enough to love anything on earth will see death as a problem. Only those who cling can lose something; only those to whom the world has given a sense of happiness know the sadness of having to die. Only those who are disappointed by longing may end up looking for death, but they just want to live properly. But this seems to be the core of all questions: When do we really live? [2]

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_9

In this chapter, we aim to use transcultural and psychotherapeutic treatment methods to show that healthy preparation for one's own death and comprehensive mourning work must be holistic. Every aspect of human aspiration and ability must be considered in order to develop a culture that embraces both life and death. It is crucial to heed the signs of death so that we can enhance our lives, as illustrated by this story from Peseschkian:

> A man befriended the Angel of Death. One day, he said to the Angel: 'You are the most successful being of all time. Wherever you go, you always reach your goal. I have a request: Let me know in good time before you come for me.' The Angel of Death agreed. One day, the Angel came to his friend and said: 'Tomorrow, I will pick you up.' The man replied, 'You can't be serious—you promised to let me know in good time.' The Angel of Death then answered: 'I gave you many signs, but you never understood my signals. When your father died, you didn't know how to interpret it. When your mother died, you didn't listen to this message. When I took your brother-in-law, your neighbor, and your friend one after the other, you closed your eyes. Come with me tomorrow.' When the Angel took his friend away the next day and brought him to heaven, the friend saw crowds of dead people crying out, 'Why didn't you let us know in time? We could have done so much beforehand.' The Angel of Death then said, 'You see now how people ignore my signals.' [3]

Many religions and cultures emphasize the connection between life and death. However, beliefs about the future after death often depend on moral, spiritual, and ideological interpretations. For example, Buddha's teachings revolve around the overcoming of suffering and death, with the promise of salvation through Nirvana. Judaism sees death as a consequence of disobedience to divine command, while Christianity views the death of a human being—as the most pivotal event in salvation history and all of world history. Especially in the question of what comes after death.

From these perspectives, we can see that the conception of life and death is closely tied to worldview and ideology. The experiences surrounding death are interpreted either rationally or spiritually, and the supernatural dimension of death remains a guiding principle in many cultures.

The fast pace and lifestyle of today's society make religious concepts about what follows death even more agonizing. Life is often focused on the future or the past, and anything that disrupts this mindset is seen as a loss. For instance, if someone loses a job after years of dedication, they may experience this as a threatening condition and a significant loss.

Similarly, young people mourn the future, hoping for something better. When this future does not materialize, fear and hopelessness arise. Loss and hopelessness lead to grief. Mourning occurs when a reality, relationship, expectation, or way of life is disrupted through separation, termination, or the irreversibility of a process [4].

It is therefore timely to explore new ways of alleviating the painful experiences associated with death and grief. The objectives of this chapter are as follows:

- To make life more lively.
- To see death not as an enemy or an illness, but as the destiny of us all.
- To recognize grief as an integral part of human development.
- Cultures and religions that propagate a holistic view of humanity can better prepare people for death, mourning, and life.

The structure of positive and transcultural psychotherapy (PPT after Peseschkian) offers the opportunity to grasp a holistic image of humanity. The four areas of conflict resolution and conflict analysis used in this therapy model will be employed here to illuminate a comprehensive approach to grief and death. Our focus will be: [5]

- Reflecting on one's own death.
- Examining case studies from various cultures and religions to compare approaches to death and grieving.
- Utilizing transcultural psychotherapy as a treatment framework for grief work.

Questions to Determine Your Attitude Toward Death and Grief

When dealing with death and grief, it is helpful to first examine one's own attitude toward death. This allows us to see how factors such as religious upbringing, cultural influences, and personal experiences shape our views. The following questions are designed to help grasp the different dimensions of attitudes toward death, from the perspective of positive and transcultural psychotherapy. These questions address various domains, such as the body/senses (e.g., death customs and bodily rites), performance/reason (e.g., burial practices), contact/tradition (e.g., cultural rituals), and future/religion (e.g., beliefs about the afterlife) [6] (Fig 9.1).

Body/Senses (Death Customs and Death Ideas)

1. What emotions do you feel when thinking about death?
   - Fear (of obliteration or annihilation)
   - Guilt
   - Meaninglessness of life
   - Determination to stay alive
   - Joy in life
   - Other (please specify)
2. Are you more afraid of dying (the process) than death itself?
   - Applies to me
   - Doesn't apply to me
3. Are you ready to die and unafraid of death?
   - True for me
   - Not true for me
   - Can't judge
4. How often do you think about your own death?
   - Daily
   - Weekly
   - Only when others die
5. If you died today, do you feel you've had a fulfilling life?
   - True
   - Partly true
   - Not true
   - Can't judge
6. Do you prepare for death through reflection?
   - Applies to me
   - Doesn't apply
7. How often do you visit cemeteries?
   - Frequently (weekly)
   - Occasionally (monthly)
   - Rarely (less than monthly)

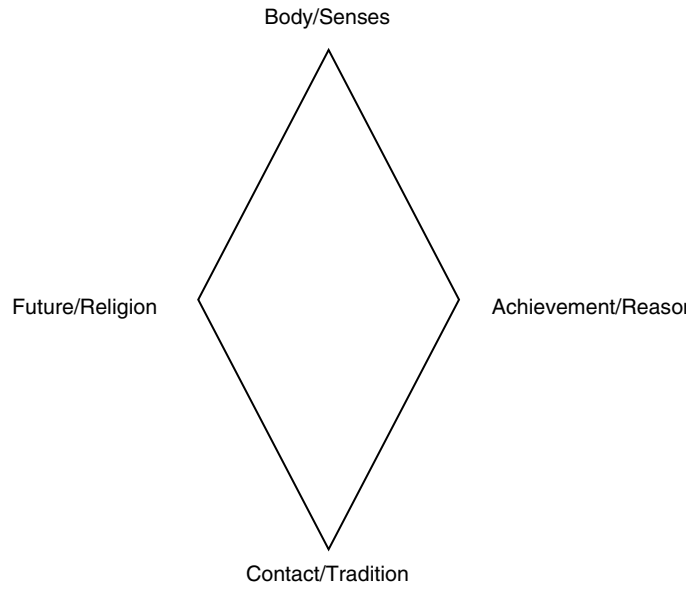


Fig. 9.1 The PPT balance model—after Peseschkian

8. How many funerals of strangers have you attended?
   - Never
   - Once
   - Several times
9. Do you watch or listen to programs about death (e.g., interviews with the dying)?
   - Yes
   - No
10. Have you learned about funeral customs in other countries?
    - Yes
    - No
11. Do you agree with these statements?
    - Reporting deaths of other nations in media: Agree/Disagree/Don't care
    - Mourning an unknown person's death: Agree/Disagree/Don't care
    - Discussing unknown deceased in family: Agree/Disagree/Don't care
12. Have you ever been near death (illness, accident, war)?
    - Yes/No
    - If yes, what were your thoughts about death?

Achievement, Performance/Reason. Burial and Death Decisions

1. What reasons might lead you to consider suicide?
   - Family problems, relationship issues, job troubles, financial hardship, incurable illness, isolation, depression, other (specify)
   - I will never consider suicide
2. How would you prefer to be laid to rest?
   - Burial/Cremation/Donate body for research/No preference
3. If you could choose, how would you want to die?
   - Suicide, sudden death, accident, hero's death, long illness, painless death, other (specify)
4. How often do you read books about death?
   - Frequently (monthly)/Occasionally/Rarely
5. Do you think death should be handled in hospitals or elderly homes?
   - Yes/No
6. Do you read obituaries?
   - Frequently/Occasionally/Never
7. How do you feel about reports of deaths in other countries?
   - Concerned/Unaffected
8. Do you visit graves of strangers in cemeteries?
   - Frequently/Rarely/Never
9. Would you attend a stranger's funeral in your community?
   - Yes/No/It's not my business
10. Why do people care less about unknown deaths compared to relatives?
    - Education/Indifference/Human nature/Other (specify)
11. Does reading about accidents or deaths make you reflect on your own death?
    - Yes/No
12. Have you ever been in mortal danger?
    - Yes/No (If yes, specify)
13. Are you exposed to death/dying in your work?
    - Yes/No

Tradition/Culture: Relationships and Death

1. Have you directly experienced someone's death?
   - Yes/No
2. Have you lost a loved one? (Select all that apply)
   - Parent, grandparent, sibling, spouse, child, friend, colleague
3. Do you agree: "As you live, so you die"?
   - Agree/Disagree
4. Would you join a discussion about death with friends or family?
   - Yes/No
5. If you had only 10 hours to live, what would you do first?
6. Do you think hospitals or elderly homes are suitable places to prepare for death?
   - Yes/No (explain)

7. At what age do people fear death the most?
   - Choose an age range or "Same at all ages"
8. Which loss would affect you most?
   - Job, property, parent, sibling, spouse, child, friend, physical abilities
9. Does your attitude toward death change as you age?
   - Yes/No
10. Are those afraid of life also afraid of death?
    - Yes/No
11. Is death discussed in your family?
    - Frequently/Occasionally/Rarely/Never
12. Do you want a religious funeral?
    - Yes/No
13. Does All Souls' Day remain meaningful?
    - Yes/Only for religious people/No
14. Whose death would affect you closely?
    - Neighbor/Stranger/None

Future/Religion, Meaning of Life, Afterlife

1. What aspect of death scares you most?
   - Body decomposition/Afterlife uncertainty/Loved ones' pain/Loss of possessions or relationships/Other
2. Would you want to know the date of your death?
   - Yes/No/Doesn't matter
3. How do you repress thoughts of death?
   - I don't repress it/I repress its brutality, unpredictability, or irrelevance to my life
4. What does death mean to you?
   - Nothing/End of existence/New beginning/Temporary separation/Salvation
5. Does life's meaning affect how people experience death?
   - Yes/No
6. If there were no afterlife, would you change how you live?
   - Yes/No
7. Does belief in an afterlife shape behavior?
   - Yes/No
8. Which phrases about death resonate with you?
   - "Gone forever," "At peace," "Closed your eyes forever," etc.
9. What shaped your attitude toward death?
   - Family losses, personal reflection, books, religion, health, media, other (specify)
10. How religious are you?
    - Very/Somewhat/Not/Unsure
11. Do you pray?
    - Daily/Weekly/Monthly/Rarely/Never

These questions were used in my research to compare attitudes toward death and dying between adolescents and adults [1]

Four Ways of Saying Goodbye from a Transcultural Perspective

Physical Dimension of Saying Goodbye or Burial Ceremony

This dimension does not focus on the biological aspect of decomposition but rather on how each culture says goodbye to the deceased in terms of the body, the senses, and emotions. Around the world, loss—especially through death—evokes emotions such as shock, which often leads to crying or other expressions of grief. In addition to crying, people in some cultures say goodbye through physical acts such as cutting hair, tattooing, kissing, or hugging. Washing the body is a common practice, and the wearing of mourning clothes signifies the new reality for those left behind. These clothes vary in color depending on culture and faith. In some cultures, dance symbolizes the connection between the living and the dead. Funeral feasts also serve as a central part of the farewell. In some cases, specific rites—such as blood sacrifices—may be performed, symbolizing a profound bond with the deceased. The rituals associated with the body and senses are deeply ingrained and difficult to abolish, as they represent a visible farewell to the bereaved.

Achievement/Performance Dimension of Saying Goodbye or Burial Ceremony

Through decisions, actions, and efforts, the bereaved honor the deceased with a proper farewell. Burial options (earth, cremation, water, air) are carefully considered, and the necessary components—such as coffins, flowers, invitations, obituaries, hearses, and funeral services—require a great deal of effort. In some cultures, building new houses or erecting tents is part of the burial process, involving a lot of time and resources. The various tasks and activities reflect the importance of the deceased, whether they were rich or poor, young or old, or held a significant social role. Funeral arrangements often mirror the deceased's societal standing.

Contact/Tradition Dimension of Saying Goodbye or Burial Ceremony

Humans are social beings who live in a culture that provides life experiences and customs. When these experiences are passed from generation to generation, traditions emerge. These traditions and rites are often applied during funeral ceremonies. For many, traditions offer a sense of security, belonging, and acceptance during grief. A show of solidarity is deeply desired and appreciated in this dimension, and many cultures emphasize the importance of being surrounded by loved ones during times of mourning.

Future/Religion Dimension of Saying Goodbye or Burial Ceremony

This dimension encompasses ideas, beliefs, and mystical thoughts about what happens after death. Questions such as "Does life continue after death?" or "Is there a reunion?" reflect the religious or philosophical beliefs of the bereaved. These beliefs shape the way death is understood and accepted. The diversity of death rites across cultures and religions testifies to the variety of beliefs about life after death. Farewell rites reflect the worldview of the individuals involved, and the hope for life after death is a common theme that can provide comfort to the grieving.

Transcultural Case Studies: Examples of Funeral Ceremonies and Rites

In this section, we present three case studies from Africa, Europe, and Asia, comparing the different ways in which families from various cultural backgrounds cope with death and grieving. We explore how these cultures integrate the four areas of positive psychotherapy—body/senses, performance/reason, contact/tradition, and future/religion—into their mourning and farewell rituals.

Case 1 from Africa

Background Information

Eke and his wife Chioma had been childless for 15 years since their marriage. Chioma suffered greatly due to this, as the entire village blamed her for their childlessness. In their community, infertility was seen as a curse or punishment from the gods. Eke and Chioma sought every possible remedy—they visited doctors, consulted medicine men and women, made donations to churches, and even sacrificed goats to appease the gods. They believed that both traditional and religious practices, whether "white" or "black" religions, should help those in need.

Chioma faced bitter rejection from other women and relatives, feeling unwanted at family gatherings. Despite her being a wealthy and capable woman, she was ostracized. Her husband, however, loved her deeply, despite opposition from his own family, who were prepared to arrange a new marriage for him.

One day, Chioma became pregnant, and the village celebrated this miracle with great festivity for a week. They offered delicacies to their ancestors and donated significant sums to the church. The pregnancy and birth went smoothly, and the

couple named their son **Iheoma**, meaning "happiness." After the birth, Chioma's status was elevated within the community—she was now addressed with respect, becoming "a Mrs." The family rejoiced, and the naming ceremony lasted three days.

Iheoma grew up healthy, attended school, and completed his secondary education. Unfortunately, Chioma was unable to have any more children.

On April 18, 1986, tragedy struck when Iheoma, along with several others, died in a shipwreck near Port Harcourt, Nigeria. His body was found after two days in the water. The news was delivered indirectly, as is customary, by three of Iheoma's friends and an uncle. The family was informed with veiled language, such as "Aka Ekpa a ga mu na zu," meaning "My left hand rises behind me," or "A man has begotten a child." Chioma and Eke received the news with shock and devastation, requiring careful supervision of the mother to ensure she did not harm herself in her grief.

Physical Dimension of Saying Goodbye or Funeral Ceremony

Iheoma's body was brought home, and the news of his death spread through the sound of drums and music. His body was taken to the family courtyard, where the women in the family prepared him through a washing ritual. Chioma was given time to carefully touch and feel her son, saying her final goodbyes in this physical way. She wept bitterly, and the process lasted 2–4 hours, with no rush. After the washing, oil and chalk were given to those involved as symbols of their connection to the deceased.

Iheoma was then dressed in his best clothing, chosen by his male relatives, while the father also took time to say goodbye. Elders delivered farewell words, and the men carried Iheoma to the front yard of the family home. These physical connections and ceremonies were essential to expressing the bond between the living and the dead.

Iheoma's body remained in the forecourt for three days, allowing the villagers to say their goodbyes in their own way, whether through touching, kissing, crying, or simply observing. Peers and relatives cut their hair as a symbol of their grief. The family hosted meals, singing, and dancing, while offerings were made to the ancestors to ensure Iheoma's peaceful transition. The parents spoke very little, choosing instead to express their grief through tears. On the third day, the final farewell was held, with relatives placing clothes in the coffin and delivering parting words.

Achievement/Performance Dimension of Saying Goodbye or Funeral Ceremony

In this village, there was no designated cemetery. Iheoma was buried in the family forecourt. His age group built tents from palm branches for the ceremony and dug his grave. Each task was performed to the accompaniment of singing. The women prepared meals, and the family had to slaughter many chickens, goats, and cows to feed the entire village. These animal sacrifices were part of various rituals performed by the men.

The tents remained standing for 40 days. During this period, it was believed that Iheoma had found his place among the ancestors. Afterward, everything, including the tents, was (were) burned.

Contact Dimension of Saying Goodbye or Funeral Ceremony

In times of death, the entire village comes together. Everyone contributes something; men bring drinks like palm wine, while women bring various foods, all of which are consumed during the mourning period. Male relatives wore special clothes, while the women dressed according to custom. Following the dress code expressed solidarity and sympathy.

Chioma, as the mother, received special attention and was required to remain indoors for seven days after the funeral. She had no contact with her husband during this time, giving her a chance to process her grief and be protected from evil spirits, which were believed to lurk in the village. The family was surrounded by support and was never left alone.

Future Dimension of Saying Goodbye or Funeral Ceremony

Many African tribes believe in reincarnation, and Iheoma's farewell ceremony was conducted with this belief in mind. Medicine men and women performed rituals to prevent him from returning in the same tragic manner. Iheoma was symbolically instructed to leave behind any factors that could cause childlessness, sudden death, or tragedy in his next life. Special goats were sacrificed, and their blood was poured into the grave to prevent the ancestors from returning. Young men painted their bodies and visited each home to drive away evil spirits.

After 40 days, all of Iheoma's belongings were thrown into the water, as he had died in the water and was believed to belong to the water spirits. The family made a pact with these spirits. During the mourning period, young men were allowed to engage in intimate relationships, with the belief that the first child conceived in the village after Iheoma's death could be his reincarnation. This custom of producing offspring for the dead is a common practice in Africa.

Grief Work

The bereaved family received support from many people over the course of several weeks and months. This communal help aided in healing the wounds of grief. During this time, Iheoma was remembered with praise, as it is forbidden to speak ill of the dead. Chioma was allowed to mourn her son for as long as she wished, although without wearing traditional mourning clothes.

Case 2 from Europe

Background Information

Karl and Ilse lived near Graz, Austria. Karl worked as a bank clerk, and Ilse was a nurse. They married at the ages of 28 and 30. Through hard work, they were able to build a beautiful home and lived happily, with both of them deeply committed to their jobs. After five years of marriage, they wished to have a child, and their desire was soon fulfilled. They had a son, Titus, who quickly became very popular with his relatives. He was baptized and received many gifts. The couple celebrated his baptism extravagantly.

Titus grew up healthy and full of energy. He was allocated his own room, where he played with friends and enjoyed the motorbikes he received as gifts, as he had a particular love for anything with wheels. At 18, Titus graduated from high school and looked forward to receiving a Yamaha motorcycle, a reward his parents had promised him for passing his exams.

On July 15, 1992, Titus traveled to Ireland with five friends. The trip was meticulously planned, and they were greatly impressed by the beauty of the country and the hospitality of the Irish people. They stayed for three weeks, keeping in constant contact with their parents. After their vacation in Ireland, the group planned to visit some sights in England. However, tragedy struck. Titus was involved in a serious accident near London. Although he survived, the accident left him paralyzed from the waist down.

He was transported home, and his parents immediately renovated their house to make it more accessible and comfortable for Titus. They spared no expense, buying gadgets and equipment to ease their son's life. Unfortunately, despite their efforts, Titus could not accept his condition. On September 28, at 11 a.m., Titus took his own life by overdosing on antidepressants while in the bath. His parents returned home around 1 p.m. and discovered his body. Ilse screamed and fainted, while Karl called the police and the family doctor. Four hours later, the funeral service agency was contacted.

Physical Dimension of Saying Goodbye or Funeral Ceremony

The family was devastated by Titus' death. They cried and mourned their son, but they managed to handle the necessary arrangements. The funeral service institution took care of all the preparations for the body, meaning there was no direct physical interaction or cleansing of the body by the family.

Achievement/Performance Dimension of Saying Goodbye or Funeral Ceremony

Karl and Ilse organized everything efficiently. They contacted all of Titus' relatives and friends and sent out obituary notices. They also composed an obituary for the newspaper, which read:

> Our son Titus was taken from us in a tragic accident. We mourn the loss of a loved one. We buried Titus in a private family ceremony. Farewell, Titus.

The pastor was informed and offered comfort, prayed with the family, and conducted the burial ceremony. The funeral company handled all other aspects of the burial. Titus' friends were not allowed to see his body, although some had wished to. After the burial, everyone visited the beautifully decorated grave. Following the ceremony, the family ate together, and the total cost of the funeral, burial, and grave was approximately 4,615 euros (66,000 ATS).

Contact Dimension of Saying Goodbye

Titus' funeral service was held with a small, close circle of family members. Guests brought envelopes containing letters of condolence. The entire funeral and ceremony lasted only one hour. Afterward, a small gathering was held where drinks were served, and lively discussions took place. Formal goodbyes were exchanged, and people shook hands with the family.

The family later received additional condolence letters from others who had read the newspaper obituary. However, nobody dared to visit them in person. Even those who called struggled to find the right words, afraid of saying something that might hurt the family. The solidarity and sympathy were felt only in spirit. After a month, life returned to normal, and the house became quiet. No one asked how the family was coping.

Future Dimension of Saying Goodbye or Funeral Ceremony

During the funeral service, the pastor read the following passage from the Bible:

> Do not let your hearts be troubled. You believe in God; believe also in me. My Father's house has many rooms; if that were not so, would I have told you that I am going there to prepare a place for you? And if I go and prepare a place for you, I will come back and take you to be with me so that you also may be where I am. (John 14:1-3)

In his sermon, the pastor spoke about resurrection and life after death. However, whether or not the family believed in these concepts of resurrection was not addressed. Ilse visited Titus' grave every week, trying to find closure and say goodbye inwardly.

The family's questions remained unanswered: Why did we buy Titus the motorcycle? Are we to blame for his death? Why did he have to die so young? Was it a punishment from God? What is the meaning of our lives now? Did we do enough for Titus after his accident? These lingering questions still weigh heavily on the parents, who have not spoken about the incident with anyone. They continue to suffer from the pain of this tragic experience.

Mourning/Grief Work

The family was left to mourn alone. They attempted to cope by immersing themselves in their work, but the grief was ever-present. Pictures of Titus filled the house, making his presence feel constant and inescapable. Every time Karl and Ilse saw Titus' friends, they were reminded of their son, leading them to tears. Discussions of death and grief remain taboo in the household, and the family continues to struggle with their emotions in isolation.

Case 3 from Asia

Background Information

Sudhir and Siva lived in Bangladesh and were devout Muslims. Sudhir worked as a government civil servant, while Siva was active in a women's movement. They already had three daughters and were hoping for a son. Siva had experienced four miscarriages, which caused anxiety with

each subsequent pregnancy. However, as Allah willed, Siva eventually became pregnant and gave birth to a son, whom they named after his father.

The arrival of their son was celebrated with great joy. The family marked the occasion with dances and songs, celebrating the birth of Sudhir. The mother's fears dissipated, and she was once again respected and accepted in her community. Sudhir grew up in a lively environment with many children and excelled in sports, especially field hockey. He was selected for the Junior State team and was chosen to compete in a championship in India at the age of 18. He trained diligently and was thrilled to embark on this journey, his first trip abroad.

On August 18, 1990, Sudhir's family, relatives, and friends celebrated and wished him success before he left for the tournament. Tragically, en route to the competition, his team's bus was involved in a serious accident. Sudhir was fatally injured, while more than 24 other passengers were on board. Some of his colleagues survived the crash, though with severe injuries. The news of Sudhir's death devastated his family, as it felt like the world had collapsed around them. Everyone mourned bitterly. According to Islamic tradition, burial must happen quickly, and so Sudhir was laid to rest that same day.

Physical Dimension of Saying Goodbye or Funeral Ceremony

Sudhir's body was brought home, where relatives and friends gathered to say their goodbyes. In their deep grief, mourners cried and beat the ground as they expressed their sorrow. The women, however, did not participate in the main farewell ceremony, as they customarily stayed in a separate room, singing dirges and weeping.

The emotional weight of the situation was especially heavy for Sudhir's mother, as she had only said goodbye to him a short while before his journey. She was surrounded by a support system of women who stayed with her. Mourners were permitted to touch the body and throw flowers into the coffin. The room was filled with a sweet fragrance, adding to the somber atmosphere.

Achievement/Performance Dimension of Saying Goodbye or Funeral Ceremony

Although body burial is traditionally preferred by Muslims, land shortages in Bangladesh have forced many families to cremate their loved ones. Sudhir was dressed in beautiful garments for his final farewell. Mourners gathered to sing funeral hymns and recite verses from the Quran. A large quantity of wood was collected for the cremation, and a fire was kindled. The Imam led prayers, and Sudhir's body was positioned toward Mecca in accordance with Islamic practice.

After the cremation, Sudhir's ashes were collected and given to his parents for burial. Following the funeral, alms were distributed to the poor, another key Islamic ritual. Many beggars and those in need received food, as the family sought to fulfill their religious duties.

Contact Dimension of Saying Goodbye or Funeral Ceremony

The death of a child brings the community together, and in Sudhir's case, a strong sense of solidarity was felt. The support extended by relatives, especially women, was significant. Siva's female relatives stayed with her for weeks, providing comfort and taking care of the household chores.

In Bangladesh, there is a saying: "A sorrow shared is a sorrow halved." This sense of shared mourning lightened the burden of grief for the family. The immense sympathy and willingness to help from the community made the loss more bearable.

Future Dimension of Saying Goodbye

The phrase "Allah has given and Allah has taken" is often cited by Muslims in times of death, reflecting a belief similar to that found in the Book of Job in the Old Testament. Muslims hold that all human events, from birth to death, are preordained by Allah. Wealth, health, life, and death are all seen as part of Allah's divine plan, and no human action can alter this destiny.

Muslims believe that those who live righteously and respect Allah's laws are rewarded with paradise. Sudhir's family found solace in

their firm belief that their son had entered paradise, having lived according to the teachings of Islam.

However, there are some Muslims who believe that the devil and demons can influence certain destinies. As such, imams are called upon to drive out these evil spirits. In Sudhir's case, his family believed wholeheartedly that his destiny was in Allah's hands and that he was now resting in paradise.

Conclusion and the Transcultural Psychotherapeutic Treatment of Grief and Mourning

Dealing with death and grief is not easy, due to the diversity and complexity of human life. Each person's definition of life and death is shaped by their worldview and lifestyle. Many questions—Why death? Why life? Who am I in life and in death?—remain unanswered.

Science and religion can offer only limited explanations. Biology, medicine, sociology, and psychology can study the how of death, but not the why. Religion, meanwhile, can only offer answers about what might come after death based on faith.

In light of this, transcultural psychotherapy provides a holistic framework for understanding death and grief. According to this model, every person has four areas—body/senses, performance/reason, contact/tradition, and future/religion—that can be used for conflict management and lifestyle development. When applied to death and grief, this model allows us to view humanity in its full capacity, making death and grief more comprehensible and manageable.

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Part III

Psychiatric and Psychosomatic Disorders

Positive Interventions in Depression

10

Fayez El-Gabalawi

Depressive Disorders: Prevalence and Definition

Depressive disorders consist of a group of psychiatric disorders in which pathological moods and related vegetative and psychomotor disturbances dominate the clinical picture. Previously, they were part of mood disorders, affective disorders, manic-depressive disorders, dysthymic and cyclothymic disorders, and they are better conceived as syndromes rather than discrete disease categories [1]. Before discussing the concept of major depression from the historical and developmental point of view, let us examine how pervasive it is as a major public concern in the United States and in the world in general.

According to the National Institute of Mental Health (NIMH), major depression is one of the most common mental disorders in the United States. For some individuals, major depression can result in severe impairments that interfere with or limit one's ability to carry out major life activities. The past year prevalence data presented here for major depressive episodes are from the 2016 National Survey on Drug Use and Health (NSDUH). The NSDUH study definition of major depressive episode is based mainly on the fourth edition of the Diagnostic and Statistical Manual of Mental Disorders (DSM-IV).

Box 10.1: Depression Prevalence

1. Past year prevalence of major depressive episode among U.S. adults in 2016 was 6.7% (female 8.5%, male 4.8%).
2. Age groups differed in their prevalence (age 18–49 was 7.4%, age 50 and older was 4.8%).
3. Race and ethnicity (Hispanic 5.6%, White 7.4%, Black 5%, Asian 3.9%).
4. Sixty-four percent of all adults with major depression had severe impairment (Box 10.1).

In fact, the Global Burden of Disease has ranked depression as the second leading cause of disability worldwide and also a contributor of burden allocated to suicide and ischemic heart disease [2]. Although direct information on the prevalence and correlates of major depression does not exist for most countries, the available data, reviewed below, indicate that there is wide variability in prevalence estimates, but that other aspects of descriptive epidemiology (e.g., age-of-onset, persistence) are quite consistent across

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_10

countries. As such, a number of consistent socio-demographic correlates have also been found across countries [3]. Prevalence of DSM-IV/CIDI major depressive episodes in the 18 countries participating in the WMH surveys showed a lifetime prevalence of major depression among high-income countries as listed in Box 10.2.

Box 10.2: WMH (World Mental Health) Surveys
France 21%, United States 19.2%, Netherlands 17.9%, New Zealand 17.8%, Belgium 14%, Spain 10.6%, Israel 10.2%, Germany 9.9%, Italy 9.9%, and total was 14.6%. Among low-middle income countries lifetime prevalence of major depression showed the following: Brazil 18.4%, Ukraine 14.6%, Colombia 13.3%, Lebanon 10.8%, South Africa 9.8%, India 9%, and Mexico 8%.

From the above survey, we see that major depression is a commonly occurring disorder in all countries where epidemiological surveys have been carried out; however, lifetime prevalence estimates of major depression vary widely across countries, with prevalence generally higher in high income versus low-middle income countries, and the age-of-onset distributions show consistent evidence for a wide age range of risk with median age-of-onset typically in early adulthood, while the course of major depression is often chronic-recurrent. Interestingly, women consistently across countries have lifetime risk of major depression roughly twice that of men; in the meantime, other socio-demographic correlates are far less consistent. Major depression is associated with a wide range of indicators of impairment and secondary morbidity, although some of these individual-level associations are stronger in high-income than low-middle-income countries.

Historical Perspective of Depression

Mental sufferings had been known to afflict humans since ancient times, and one of the common pervasive conditions was known as Melancholia (depression). Until the Greco-Roman period, the condition was frequently attributed to demonic possession, for which many harsh measures, such as exorcism—and possibly trepanation in prehistoric times—were used to rid the person of the evil spirit. However, during the fifth century B.C., Hippocrates had explained mental illnesses as resulting from imbalanced body fluids (the four humors were: yellow bile, black bile, phlegm, and blood) and that depression was caused by excessive black bile in the spleen. In the seventeenth century, Robert Burton wrote his monumental work "Anatomy of Melancholy" in which he described multitudes of clinical symptoms of Melancholia from the sufferer's perspective that seemed to resonate with our current understanding of depression. And interestingly he proposed Burton's six non-natural things that referred to such environmental factors as diet, alcohol, biological rhythms, and perturbations of the passion such as intense love.

The modern conceptualization was introduced by the French psychiatrist Jean-Philippe Esquirol (1772–1840) who suggested that a primary disturbance of mood might underlie many forms of depression and related paranoid psychoses and the symptoms were the expression of the disorder of affections (affective disorders), the term was coined by the British psychiatrist Henry Maudsley (1835–1918). However, Emil Kraepelin (1856–1926) described manic depression illness and he believed that endogenous affective disorders are somatically caused, yet he conceded that occurrence of psychogenic states of depression (due to loss or misfortune) is psychologically caused. It was Adolf Meyer (1866–1950) who bridged the gap between psyche and soma, emphasized the individual personal history and biographical factors, and introduced the term "psychobiology" emphasizing both biology and psychological causes of depression and other

mental illnesses. So finally a conceptual shift from reductionist to pluralistic causation of depression was achieved [1].

The Contemporary Etiological Models of Depression

Etiological frameworks for depression included the biological models—including genetic and evolutionary, psychoanalytic, and psychodynamic behavioral, and cognitive models.

Biological Models

The biological model hypothesizes a connection between depletion or imbalance of biogenic amines such as catecholamine (norepinephrine), indoleamine (serotonin), and clinical depression. Current genetic evidence indicates a possible predisposing factor, however it is not known exactly what is inherited. An evolutionary model to explain depression proposes that our brains have evolved with a negative-event bias that overestimates threats as a strategy to avoid dangers and enhance survival chances.

Psychological Models

Psychological models include aggression turned inward model (Abraham-Freud), object loss model (Freud-Bowlby), and a loss of self-esteem model. Cognitive model (Aaron Beck) hypothesizes that negative attributional styles such as thoughts of being helpless, unworthy, and useless can generate biased interpretations of life events. Learned helpless model (M. Seligman) proposes that depressive disposition can develop as a result of repeated past experiences of uncontrollable helplessness.

The Role of Positive Psychiatry

From the above discussion, it is evident that the etiological models of psychopathology of depression are inconclusive, and there is not yet a clear genetic, biological, or psychological/social marker for depression. Moreover, the rapid rise and widespread use of antidepressant treatment of depression, based on the biological model of depression, has not been without controversy. Antidepressants are helpful in about half to two-thirds of cases, especially with severe depression, however in most cases the effect is modest [4]; and in many cases especially in youth with major depression the antidepressant effect is close to a placebo effect. It is important to point out that clinical studies of antidepressants have primarily focused on reducing the symptoms of depression and not on a broader range of potential outcomes (such as changes in everyday functions, cognitive abilities, quality of life, etc.).

An integrative model is most likely the approach to understand and treat depressive illnesses. Thus, positive psychiatry opens the possibility that psychopathology is not the driving force to understand and treat depression, instead it emphasizes hope, optimism, character strength, and gratitude, among others, which can act as powerful protective factors against genetic vulnerability and precipitating stressors of depression.

A study on the effectiveness of six internet-based Positive Psychology interventions concluded that two—*using signature strengths in a new way* and *three good things*—increased happiness and decreased depressive symptoms for 6 months. The other effective interventions *The Gratitude Visit* led to positive changes for 1 month [5]. In a large placebo-controlled clinical trial, where a quarter of the treated sample had depression, Positive Psychotherapy (PPT after Peseschkian) was shown to be an effective interdisciplinary approach with long-term symptom reduction [6]. Finally, a meta-analysis looking at Positive Psychology Interventions concluded that they enhance wellbeing and decrease depressive symptoms and recommended clinicians incorporate positive psychology techniques "particularly for treating clients who are depressed" [7].

Clinical Application

A.R. is a 39-year-old white female attorney, married with one child. She was referred to outpatient treatment following her 3-week hospitalization on inpatient psychiatric unit for the treatment of severe recurrent major depression with suicidal ideation. That was when she met her current outpatient psychiatrist in the year 2014. The psychiatrist learned that the hospitalization was her third one, and that her history of clinical depression started during the year of 2011, and that she had been hospitalized approximately once a year since the onset of the illness. Her hospitalizations were always ushered in by sleep disturbance characterized by difficulty staying asleep, lack of motivation, decreased energy level, decreased appetite, and a sense of hopelessness and helplessness associated with fleeting suicidal ideation without intention or plan. She had never attempted suicide and considered it to be unfair to her child and her supportive husband. She had been tried on many antidepressants at different times alone and sometimes in combination with mood stabilizers as augmenting agents with varying results. The last medication regimen included Bupropion extended release, Lamotrigine, and Trazodone.

In our first session, AR conveyed clearly that she had been tired of all those medications that seemed partially effective and frequently associated with side effects. She was a strong believer in modern pharmacology and the power of medication to correct the "chemical imbalance" that she believed to be the cause of her depression. Her disappointment and disillusionment in the curing power of medications was increasing. AR was convinced that her depression is hereditary, since her mother also suffered from major depression and mother's response to antidepressant treatment was generally favorable.

AR grew up in a small town in Pennsylvania, the oldest of three children, a sister 3 years younger, and a brother 5 years younger than her. Parents had frequent marital and financial problems due to father's drinking problems and they were eventually divorced when AR was a junior in high school. She put all her energy into excelling in school and tried to help her mother raise the two younger siblings. Due to the difficult financial situation, she had to work part-time while finishing college and went on to study law. She graduated from law school with a large debt, yet was motivated to start working immediately to pay her loans and to help her family financially. She joined a financial law firm in which the work environment was stressful and demanding, she worked long hours and always felt that she could not keep up with supervisor's expectations. She met her husband shortly after joining the law firm and had her only son who was 7 years old in 2014. AR described her husband as a caring and very supportive person. Because of the stressful work and the long hours AR had to spend working, her husband, who worked in the computer business, began to do most of his work from home, which made him available for childcare. She was the main breadwinner of her family and always felt obligated to work hard to secure a decent life for her immediate family; however, it was at the expense of the time spent with her husband and son.

In addition to the hospitalizations, AR had occasional therapy sessions with different therapists and was seen by psychiatrists for medication follow-ups; however, the outcome was not favorable since she continued to struggle with periods of depressive symptoms and three admissions.

In reviewing the history of treatment and medication with AR, it became obvious that the focus of most treatment modalities had been on addressing her psychopathology and deficits and attempting to correct them. Whether the problem is a chemical imbalance that needed to be corrected by antidepressants and other somatic agents, or the problem is negative and dysfunctional thoughts that needed to be corrected through cognitive behavioral therapy sessions. Or the problem stemmed from her early formative years when she was exposed to parents' frequent discord due to father's drinking problem that culminated into their divorce and father's final abandonment of the family, which may have left AR with a deep feeling of poor self-worth and low self-esteem. Although those were

legitimate issues that probably played a role in her depression, and convinced AR that she is somehow a deficient person, and she became even more despondent that all those treatments had never achieved the goals of correcting the problems.

It was clear that AR history of psychiatric evaluations and treatments, that was based on psychopathology and deficits will always recommend specific treatments, such as medications and individual therapy, while an evaluation that is based on the principles of positive psychiatry and wellness will produce different treatment recommendations that enhance the quality of life [8, 9]. It seemed that another side of AR as a person was never explored or utilized and that a whole different mental state and way of thinking had to be adopted and tapped on, which could drastically change the direction of treatment and lead to a positive outcome.

Positive Psychiatry Treatment Approach

Assessment and Opening Sessions

The initial phase of the treatment consisted of first, addressing certain beliefs about what it meant for AR to have depressive illness; second, involving the husband to some degree in her treatment; and finally, identifying positive traits and strength that she had. A discussion ensued regarding AR conviction of having a hereditary chemical imbalance, in which the psychiatrist explained that the term "chemical imbalance" could be misleading and inaccurate, and it is only a hypothesis, no chemicals are routinely measured. The relationship between the chemicals (neurotransmitters) and depression is very complex –discussion continued—and it is not a causal relationship, meaning the presumed deficiency could be a cause or, in some cases, an effect of depression, also the effect of antidepressant on brain chemicals is most likely through several intermediary steps which are not fully understood. In terms of heredity, the psychiatrist explained, it is not known what exactly that is inherited in addition, genes are not operating in isolation, genetics are in constant interaction with the internal and external environment, genes can affect behavior and behavior can affect gene expression (epigenetics). And it is better to conceive of hereditary as only a possible vulnerability that can be reduced through modifying stressful factors.

Building a Support System

The second step was to involve the husband in her treatment, since her depression had affected the family and her stressful work had limited the time the family could spend together. AR agreed for the husband, who is a caring and supportive person, to join in every other session for the first year.

Identifying and Using Character Strengths

Identifying strengths was the third step, AR was encouraged to take the VIA survey available online, in which she showed high character traits such as kindness, judgment, and gratitude; she also showed a reasonable level of zest, a modest sense of hope, and a good level of love. She was encouraged to be cognizant of her strength and to nurture her sense of hope and optimism during most of her daily activities.

Addressing Stressors

During several sessions that included her husband, and after long discussion regarding her stressful work environment, AR reached a decision to quit her job and work as a part-time lawyer in a low-stress firm and to accept the less income yet she will enjoy more time with her husband and son and time for wellness. That also allowed her to volunteer a few hours per week as a legal advocate for mentally ill patients, an activity she always wanted to do but could not due to her busy schedule in the past. The advocacy for

patients made her highly conscious of the value and the power of *optimism and hope* that she began to instill in her clients. AR had been interested in mindful-meditation in the past but could not find the time to practice, now she joined a group for mindful-meditation, and began to do her reading and meditation practices for 20 min daily. She added to her activities regular walking daily for half-hour. Within a few months from adopting a positive psychiatry approach, sleep normalized, mood has improved significantly and her outlook on life has changed positively. Medication regimen has been simplified, no mood stabilizers or sleep medication were required, and she only needed half of the dose of bupropion antidepressant. She had achieved remission for longer than 2 years and most importantly, her quality of life and well-being have been enhanced.

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11 Positive Interventions in Anxiety Disorders

Salman Majeed, Rabia Salman, Patrick Lau, and Consuelo Cagande

Introduction

Ancient Greek and Roman physicians and philosophers differentiated anxiety from other negative affects and labeled it as a medical disorder [1]. In the collection of Greek medical texts known as Hippocratic Corpus (dominant from 460 BC to 370 AD), a phobia of a man named Nicanor is described. Nicanor experienced severe persistent anxiety on hearing the voice of the flute at night time [2]. This typical phobia was labeled as a medical disorder.

Roman Stoic philosophers Cicero and Seneca described the clinical features and the cognitive treatment of anxiety that have much overlap with modern cognitive psychotherapy. In the *Tusculan Disputations*, Cicero (106 BC to 43 BC) classified affliction, worry, and anxiety as disorders, and made a distinction between pathological anxiety and sadness [3]. He also distinguished between trait anxiety (*anxietas*—the predisposition to anxiety), and state anxiety (*angor*—current anxiety). Seneca (4 BC to 65 AD), in his book *Of Peace of Mind (De tranquillitate animi)*, discusses the need to focus on the present rather than the future in the treatment of anxiety disorders—which is a foundational principle of modern Mindfulness practice.

Epicurean Philosophy founder, Epicurus (341 BC to 270 BC), also similarly noted that to reach the state of tranquility and to be worry free (Ataraxia), one needs to place attention on the present rather than having negative thoughts about the past or future.

In 1621, Robert Burton published a comprehensive review of the literature from Classical Antiquity up until the seventeenth century in *The Anatomy of Melancholy*. In this work, Melancholia included both depression and anxiety; the diagnosis would be applied to individuals with negative affect or internalizing symptoms. Boissier de Sauvages (1706–1767) worked on classifying diseases and published the first major French medical nosology. He used the term Panophobia to describe anxiety. His further subdivisions of Panophobia overlap with our current anxiety disorder subtypes. In 1869, George Miller Beard describes neurasthenia, which continues to be a diagnostic term today. Neurasthenia includes symptoms of anxiety and depression in addition to weakness, dizziness, and fainting. Freud initially considered anxiety to be the physiological buildup of libido but later redefined anxiety as a signal of threat in the unconscious.

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_11

Current Understanding of Anxiety

Anxiety is a universal, evolutionary, and adaptive phenomenon. With a radar-like threat detection system, the thalamus and the amygdala can activate the autonomic nervous system, hypothalamic-pituitary-adrenal (HPA) axis, leading to the release of adrenaline and cortisol which maximize the fight or flight response for survival. In anxiety disorders, this threat detection system is sensitized to various fearful stimuli and gets activated in the absence of objective threats to survival.

Anxiety disorders constitute the most common psychiatric disorders, affecting almost one in five adults in the USA [4]. Estimates are that 25–30% of the population experience an anxiety disorder at some point in their lifetime, leading to an enormous disease burden and high healthcare costs [4, 5]. According to the National Comorbidity Survey (NCS), the lifetime prevalence rate for any anxiety disorder was 30.5% for females, vs. 19.2% for males [4]. The prevalence of self-reported anxiety has been increasing as well [6] (Box 11.1).

Furthermore, anxiety disorders have a chronic course of illness [9].

Box 11.1 All of the Anxiety Disorders Have a High Lifetime Prevalence

- Panic disorder 5.7%.
- Generalized anxiety disorder 6.2%.
- Agoraphobia 2.6%.
- Social anxiety disorder 13.0%.
- Specific phobia 13.8%.
- Obsessive-compulsive disorder 2.3%.
- Post-traumatic stress disorder 1.3–12.2% [5, 7, 8].

Biological Etiology

The survival value of anxiety facilitates genetic selection of anxiety genes because of evolutionary advantage and genetic inheritability; this can explain the significant genetic predisposition of anxiety disorders. Inheritance of shorter alleles for Serotonin *ss* (two short alleles) is directly involved in the genetic inheritance. A study on young rhesus monkeys showed that brain function and metabolism is passed on to the offspring from anxious parents, giving the progeny a higher risk of stress-related psychopathology [10]. Also, disruptions in neuroendocrine and neurotransmitter pathways have been associated with anxiety disorders, which have provided great implications for the psychopharmacological treatment of these diseases [11].

Genes often interact with their environment to make phenotypes appear, and some of these epigenetic changes are transmitted to the next generation as well.

Psychosocial Etiology

Ancient Greek philosopher Epicurus, and in modern times Freud, noted that seeking pleasure and avoiding pain drives human behavior [12]. Anxiety disorders, through their repetitive, intrusive, and unwanted activation of the threat detection system forces one to avoid fearful stimuli, no matter how irrational it might be. These psychological processes start from infancy and continue as we grow to make sense of the world and develop various mental schemas. As the infant individuates into their self and realizes their separate existence, developmentally normal separation and stranger anxiety starts. The brain detects danger when the caregiver disappears, or an unfamiliar person shows up. At age two or three, most children outgrow separation and stranger anxiety as they realize that either condition by itself does not pose any danger. Those born with a genetic predisposition, or through a combination of Nature and Nurture, continue to experience significant clinical anxiety. Bowlby [13] argued that children who suffered neglect and uncertainty develop an anxious-ambivalent attachment with the caregiver and do not overcome the fear of separation or abandonment [14]. Bowlby also noted, on the other spectrum, overprotection also led to anxiety disorders [13].

Within the normal developmental range, children continue to experience anxiety from various real or imagined objects, for example: being alone, dark, animals, thunder, monsters, etc., through misattribution and projection.

> All around the house is the jet-black night;
> It stares through the window-pane;
> It crawls in the corners, hiding from the light,
> And it moves with the moving flame.
> Now my little heart goes a-beating like a drum,
> With the breath of the Bogie in my hair;
> And all around the candle, the crooked shadows come,
> And go marching along up the stair.
> The shadow of the balusters, the shadow of the lamp,
> The shadow of the child that goes to bed –.
> All the wicked shadows coming, tramp, tramp, tramp,
> With the black night overhead.
> ROBERT LOUIS STEVENSON, *A Child's Garden of Verses.*

Through the psychodynamic lens, clinicians might be able to identify various deep-rooted anxieties. Disintegration anxiety indicates excessive fear that the self will fragment because others are not responding with needed validation. Persecution anxiety refers to fear that self is getting invaded and annihilated by an external evil force. Fear of loss of the object, or the loss of the love/approval of an object (parent), or guilt from harsh superego of not living up to the internalized standards constitute other anxieties.

Behaviorists, on the other hand, explain anxiety as (1) a conditioned response to previous painful experiences, e.g., past trauma inducing horror from the perpetrator and resulting overgeneralized difficulty in developing trust, or (2) to result from overprotective anxious parenting as indicated by Social Learning theory. Anxious parents can often enable anxious behaviors in children by inadvertently teaching them that the world is dangerous and that it is better to stay home and not to take any risks. They may also learn to mimic the behavior of their parents and others after observing them according to the social learning theory [15].

Existential theorists indicate the anxiety to stem from (1) the fear of annihilation, (2) experiencing a void in existence and meaning of life, or (3) a guilt or condemnation that one has not lived to the moral standards [16].

The Cognitive Model describes anxiety as stemming from certain faulty cognitive processes that make the person experience heightened vulnerability [17]. Through fearful information processing/appraisal, anxious individuals overestimate the threat and underestimate their ability to cope with it. Common cognitive processing errors include:

1. Catastrophization (thinking that mistakes would have a disastrous outcome).
2. Magnification (viewing shortcomings to be significant flaws).
3. Selective attention (over scanning the environment for threats while missing the safety cues).
4. Personalization (wrongly blaming themselves for adverse events/outcomes).
5. Discounting the positive (not taking credit for excellent results).
6. Overgeneralization (generalizes an adverse event to oneself or the environment).
7. "Should" statements (overly self-critical).
8. Control fallacies (believing one has no control or complete control of everything).
9. Minimization (underestimating one's resources).

When faced with the fearful situation and stimuli, anxiety increases significantly with an automatic fight, flight, or freeze response and resulting physiological symptoms. Withdrawing or avoiding the trigger removes that painful anxiety response which is behaviorally reinforcing. Over time, avoidance behaviors perpetuate the vicious cycle of worsening anxiety and decrease the ability to cope with the fears [18].

Larger community also provides positive and negative social experiences. For racial minorities, exposure to discrimination leads to a higher risk of anxiety, but nativity is protective [19]. Other negative interpersonal experiences, for example, bullying also increase the vulnerability for anxi-

ety disorders. Crime in the community leads to fear, resulting in a high likelihood of anxiety as well [20].

Positive Psychiatry Applications

Positive Psychiatry does not replace the existing treatment, and instead broadens the scope of practice to health promotion and centers the treatment framework to identify and strengthen Positive Psychosocial Factors (PPSFs) Box 11.2.

Box 11.2 To Treat Anxiety Disorder, Positive Mental Health Clinicians Must Understand, Assess, and Strengthen the Positive Psychosocial Factors (PPSFs) of the Following

- Optimism.
- Resilience.
- Perseverance.
- Positive emotions.
- Positive appraisal.
- Positive selective attention.
- Positive internal coping skills.
- Problem-solving skills.
- Humor.
- Emotion regulation.
- Facing one's fears.
- Spirituality.

Positive psychiatry extends beyond an individual approach and involves family and community work. For example, a Child & Adolescent Psychiatrist providing an individualized plan (with strengths and challenges) to the child, family, and school models the Positive Psychiatry approach. Parent training work, family meetings, and discussions with school counselor/teacher not only treats the identified child patient but also benefits other siblings and students.

Positive Psychiatry approach helps clinicians utilize the client's strengths to facilitate optimal exposure.

Therapeutic Relationship/Stance

Positive Mental Health practitioner indeed approaches the patient as a person gifted with unique strengths. Clinician actively elicits and listens to the existing strengths which will be utilized to treat the client's vulnerabilities and adversities. Provider actively communicates the acceptance, respect, and belief in their patients' capabilities. Therapist and patient jointly co-investigate the identified problems through collaborative empiricism [21], as in two scientists collaborate to solve a problem. The patient retains rather than strengthens their sense of agency in the treatment.

It is important to highlight that this therapeutic stance is not neutral and is rooted in positivity instead. Also, it is not a forensic or investigative stance to regularly scan if the patient is telling the truth. Instead, clinicians listen to the client with an accepting and believing position. If there is a discrepancy between one's account or through a collateral source, clinician frankly discusses that in a matter of fact tone to understand the situation better. The therapist is not searching for a confession and instead facilitates their patients to start experiencing their authentic self in that relationship without fearing judgment.

Here, we represent a few tools that clinicians could use in treating anxiety disorders. Many of these tools have been described through various psychotherapeutic and philosophical traditions, some like mindfulness from the time of Stoics.

Exposure

> Each of us must confront our own fears, must come face to face with them. How we handle our fears will determine where we go with the rest of our lives. To experience adventure or to be limited by the fear of it. –Judy Blume

Whether some therapeutic approaches start with exposure or get there overtime, in the end, one overcomes fears by facing them. Gaining an understanding of the pain and purpose of exposure will be productive in overcoming anxiety through the development of the ability to persevere.

Mindfulness

Anxious ruminations involve either replaying the past negative situations and interactions or dreading the future occurrences. Mindfulness allows one to be in the present moment. From the time of Stoic and Epicurean philosophers, mindfulness has been described as a way to achieve happiness and tranquility in life. Current research has validated the effectiveness of mindfulness for the treatment of anxiety [22].

Mindfulness is a non-judgmental and accepting awareness of the present. One could observe thoughts in a de-centered and non-attached manner [23].

Cognitive Restructuring

Cognitive therapy teaches how to correct the cognitive errors responsible for anxiety. Understanding that anxiety centers on experiencing heightened vulnerability, with more attention to threats, and less to the safety cues and resources, cognitive restructuring helps reappraise the risk more accurately and improves the confidence on internal and external resources.

Reflective Thinking

Anxiety reduces rational, reflective thinking as more primitive centers of the brain (Amygdala) take over. Engaging in more reflection, description, and writing activates more rational parts of the brain (Cerebral Cortex) which helps to challenge one's negative thoughts and enhances strategic thinking.

The Sense of Agency

Anxiety disorders, with a sense of heightened vulnerability, induce the feeling of helplessness. Self-efficacy and outcome expectancy are directly correlated with the ability to handle anxiety [24]. Individuals with an internal locus of control perceive life circumstances to be under their control, vs. those with an external locus of control think outside forces control them—hence increasing the sense of helplessness. Patients could train themselves to experience improved perceived self-efficacy.

The related concept of Personal Mastery—the power of "I can" empowers and motivates one to face and overcome threats. A fundamental assumption is that human beings need the motivation, perseverance, expectation of success, and positive attitude to make a specific behavior happen. Both Personal Mastery and perceived self-efficacy correlate positively with positive mental health outcomes.

Letting Go

With increased helplessness, anxiety forces one to control as many variables as possible. Contrary to this anxious pressure, treatment targets letting go of control and negative emotions. Learning to live without the safety of having control of the environment would significantly reduce the amount of anxiety experienced, preparing individuals for situations where they cannot obtain control. Mindfulness could help achieve this skill of letting go of negative emotions.

Letting go of perfectionism and particular validating statements from others would also reduce significant pressure. The University of British Columbia Cognition Inventory—"Letting Go" provides a useful measure of the process of letting go. Its advantage over other scales is that it measures multiple negative thought themes separately, accounting for the fact that different people do not fixate on the same things [23].

Resilience

Resilience is the ability to bounce back from adversity. Resilience competencies could be improved and taught. Resilience has an inverse correlation with the risk of PTSD [25–27]. The concept of resilience can thus be incorporated into the treatment of PTSD [28]. Some individuals become stronger when they experience

adverse events in their lives, exhibiting posttraumatic growth.

Resilience includes six core competencies:

1. Self-Awareness (awareness of one's thoughts, emotions, and behaviors).
2. Self-regulation (ability to regulate one's thoughts, feelings, and behaviors).
3. Optimism (hope, positive outlook).
4. Mental Agility (cognitive flexibility, taking others' perspective, adapting).
5. Character strengths (identifying major strengths and using them to overcome weaknesses).
6. Connection (healthy connecting with others, compassion, seeking/offering help).

Optimism is a particularly important skill for treating anxiety. With more sensitive threat detection, anxiety biases one to anticipate the high risk of various choices. When patients do not achieve the desired outcomes from a treatment plan within a short amount of time, they may experience discouragement if they do not have optimism to support them.

Self-regulation could be taught through various methods. Biofeedback could also be utilized to help patients learn to control previously involuntary body processes [29]. For those with anxiety, patients would monitor their heart rate, respiration rate, skin surface temperature, and heart rate variability as they control their sympathetic arousal [30]. With this concrete and visible information, patients could learn to regulate themselves more easily. In a four-week study of anxious graduate public health nursing students, biofeedback has been shown to reduce stress and anxiety [31].

People who identified and used their character strengths reported greater vitality, self-esteem, and positive affect as well as reduced perceived stress [32].

Healthy Sleep

Sleep can contribute to and worsen mental health issues, including anxiety disorders [33]. Improving sleep on its own makes a significant impact on quality of life. Regular circadian rhythm affects physiological processes in the body, including metabolism and hormone regulation which leads to improved sleep and higher energy levels through the day [34]. Ways to promote healthy sleep include decreasing alcohol, caffeine, and nicotine consumption, keeping the bedroom dark and free of distractions, only using the bed for sleep and sexual activity. In more severe cases of insomnia, medications may be utilized as well. Furthermore, providers can work to improve patients' circadian rhythms through light therapy and the utilization of melatonin.

Exercise/Physical Activity

Through the involvement of biological as well as psychosocial processes, healthy exercise helps reduce anxiety. Jayakody et al. conducted a systematic review to examine eight randomized controlled trials for the effectiveness of exercise in reducing anxiety symptoms [35]. It revealed that exercise does help to alleviate symptoms to an extent, and would likely work best as an adjunctive treatment. Anaerobic and aerobic exercise did not differ in terms of their results, and the type of intensity did not change the outcomes of the exercise as well. Both physiological and psychological mechanisms explain the reason why exercise can help with anxiety disorders [36]. Aerobic exercise, when performed regularly, has been shown to decrease sympathetic nervous system and hypothalamic-pituitary-adrenal axis reactivity; both of which have been implicated to be relevant in the pathophysiology of anxiety disorders. Exercise also provides exposure to physiological changes that can promote anxiety, increasing tolerance for these symptoms. Furthermore, exercise helps to build self-efficacy; it creates the perception of having control over potential threats. Similar to meditation, exercising can act as a distraction technique for reducing anxiety.

Spirituality and Religiosity

Religious and spiritual practices have an association with improved health outcomes and reduced anxiety [37]. With his systematic review, Koenig

concluded religion and spirituality were associated with significant mental health benefits, including a reduction in anxiety. Perhaps, transcendent beliefs provide hope and a deeper meaning of life or the presence of a higher being. The benefit may also come from social support or the sense of belonging that often comes from organized religion [33].

Family Engagement/Parenting

Family therapy is highly effective and yet underutilized for the adult population. Engaging the family and parents as allies in treatment increases the success rate. Rather than discussing anger on parents and their shortcomings for months to years, it is far more useful to bring them in the session and help them connect as therapist mediates those conflicts. Clinicians must be sensitive to parental guilt of anxious parents who commonly blame themselves for their child's anxiety. Anxious parents have the unique strength of the first-hand experience of their child's illness. While anxious parents cannot control the transmission of specific susceptibility genes, they can help minimize the increased psychosocial risk factors (as noted above) for their children. Child's anxiety could be a strong motivation for parents to seek their treatment. Extensive psychoeducation for the anxious parent and child should include the purpose of going through the pain of exposure and long-term cost of the avoidance behaviors. Involving the non-anxious parent or family member in a supportive manner could reduce marital conflict and increase the chance of treatment success.

Social Support

Secure, healthy interpersonal relationships significantly increase one's ability to cope with anxiety and prevent isolation. Types of social support include: emotional (listening and validating), instrumental (providing tangible help), informational (giving advice), and feedback [38]. A cross-sectional study revealed a significant association between perceived social support and decreased anxiety; the support can come from spouses, family members, friends, and healthcare workers [39]. Also, in a primary care setting, a study showed that increased perceived social support amplified the effects of treatment [40]. Social and interpersonal skills could be taught to anxious patients. Healthcare providers themselves act as a source of support through active listening and appropriate validation, giving emotional support to the patient.

Social-emotional curriculum at schools, bullying prevention programs [41], and racial/ethnic/religious tolerance can be global community-level prevention efforts.

There is a significant need for more research to expand our understanding of mental health promotion and utilizing positive psychiatry approaches.

Clinical Case

Veronica is a 21-year-old college student in her senior year, with a perfect academic record, and is well-liked by most classmates. The college counselor has referred her to the psychiatrist for further assessment. Veronica has been dealing with severe anxiety for years that she has successfully masked from everyone, without ever asking for help. She has always been a "perfect" student with no complaints from anyone. Veronica worries excessively to not "mount to anything," and be an embarrassment for her family. Recently, her classmates have noticed her hands shaking, and highly aware of her shakiness and palpitations; she has been finding it even more challenging to interact with others. Veronica fears to make serious mistakes and appearing incompetent in front of others. Despite her perfect grades, she perceives any small corrections or suggestions by teachers as gross incompetence. She describes feeling exhausted by these constant worries to the point she wishes she "just disappears." She denies any suicidal thoughts and finds suicide to be against her religion. She feels helpless to stop her anxiety and is skeptical if treatment would be useful.

Veronica avoids social interactions and small talk. She finds it more comfortable to stay by her-

self despite her desire to connect with others. Veronica finds it hard to stop thinking about any negative interactions, sarcastic jokes, and passing remarks. She tends to apologize excessively, which seems almost involuntary for her. Veronica has had trouble sleeping and frequently feels exhausted. She tends to worry about her family and her parents' health. She has been excessively worrying about not getting desired internship or entry-level job that "everyone else in her class has already gotten by now."

Her father is a computer software engineer, and mother an accountant. Their jobs keep them busy so they do not socialize much. However, they tend to stay quiet even if they are outside the home as a family. Veronica believes her mother has anxiety, but she has not been diagnosed or treated. Veronica is the youngest of three children; her siblings are successful professionals and were also high achievers at the top of their classes through the school. Her parents have always reviewed her grades rigorously and compared her performance with others. While Veronica has always made it onto the honor roll, she does not feel that her parents have ever been proud of her.

Veronica does not recall any significant traumatic incidents. She had three close friends in school, but she did not socialize outside of school and has lost contact with them after graduating from high school. She has not had any romantic relationships.

On strength assessment, Veronica exhibits *high persistence, kindness, and self-regulation*.

A therapist and a psychiatrist collaboratively treat Veronica for Generalized Anxiety disorder and Social Anxiety disorder. They appreciate her perseverance and pushing through the challenges over the years by herself, how despite constant dreads and fears, she has excellent academic achievements—successes that Veronica does not think much of.

With this accepting/appreciating stance, they provide her with extensive psychoeducation about her anxiety and its physical symptoms of palpitations and tremors associated with neuroendocrine connections. She can now connect these details with her previous awareness of adrenaline and fight or flight response. The team provides her with realistic optimism and collaboratively devise a biopsychosocial treatment plan with her. Although interventions at all start together, below (non-chronologically) we break down the process.

For the biological component, the psychiatrist explained her physiological symptoms of tremors and palpitations. Also, Veronica routinely missed the breakfast and most days she only at one meal with a couple of snacks other times. The team discussed the need for healthy and balanced nutrition. She did not have specific eating disorder cognitive distortions but did not give much importance to regular meals. She agreed that she would start at least two meals and will have two snacks regularly.

The psychiatrist discusses the circadian rhythm and the importance of healthy regular sleep with her. During the discussion of bedtime, Veronica shared that many nights she studies until 2–3 am. Together, they discuss planning her day differently so she could get healthy sleep.

Veronica was already thinking that she should start exercise but felt anxious that others would judge her in the gym. The team discussed the value of exposure and the long-term benefit of facing these fears of judgment to overcome the anxiety.

The psychiatrist discussed the risks, benefits, and alternatives of SSRIs and provided her with printed material to review as well as talk with her parents if any family member has responded better to any of these medications. To her surprise, Veronica learns that her mother has been managed successfully on Sertraline. As patients with anxiety disorders typically respond to the higher dosages, but side effects/tolerability is better managed through titration, the plan was discussed with Veronica. She is also well informed of the delay in the therapeutic effects and multiple choices available, so she does not prematurely lose hope in the treatment.

Psychological interventions start from the moment of history gathering. Veronica understood the importance of exposure and viewed small talk and social interactions that she used to dread as more purposeful. Veronica had briefly

learned about meditation yoga in her classes, but found it hard, "I can't keep my thoughts focused." The therapist normalized the floating of thoughts and encouraged to observe them floating, and bring the attention back slowly each time. Veronica found it helpful that "there is no wrong way of doing mindfulness," and the whole practice involves acceptance and non-judgment.

The therapist helped her understand the anxiety as an internalizing disorder and ways to challenge anxious thoughts. She also introduced the concept of cognitive distortions. Through her curiosity and love for learning, Veronica started identifying the instances where she tended to process information from the anxious standpoint and was able to engage in more strategic reflective thinking. She was proud to show her journal where she had challenged her initial response and shifted her fearful reactions in a more thoughtful deliberate manner. The therapist deeply appreciated her courage, and Veronica laughed that "I'm not discounting my positives" and was able to take the compliment.

Veronica experienced distress in expressing her opinions to others, notably in case of a conflict. The therapist provided her with education about assertiveness and various methods of communication. She was afraid to offend anyone. The therapist did a few role-play exercises during the meeting and encouraged Veronica to find opportunities for assertiveness during the sessions with her. Veronica's inhibited communication also appeared closely linked with her perfectionism and fear of making mistakes. Together they worked on letting go, being imperfect, and the inevitability that like everyone else, she will—and can—make mistakes, and that will be alright.

Veronica engaged in significant social changes herself, as she also enrolled in volunteer work for the freshman class. She was able to observe other anxious students, and the same compassion that she experienced for them, she was able to extend to herself.

One day, Veronica asked the therapist that her parents are coming into town next week and if they could join her session. The therapist welcomed the idea. The family session turned out to be highly productive, as her parents for the first time opened up about their own vulnerabilities and emotional communication difficulties. Veronica was in tears to hear that they have always been proud of her and from their standpoint they were taking an interest in her education and peers. Both parents grew up in traumatic environments and were focused on providing safety for their children. They became tearful while expressing their guilt in not understanding their daughter's emotional states. With her oozing compassion, Veronica jumped to comfort them as she made a different sense of years of experiencing falling short of their expectations.

Summary

Anxiety disorders have been recognized since ancient times. Over the centuries, we have learned to conceptualize these disorders from its biopsychosocial aspects. As through the traditional disease model, we have extensively studied the challenges and problems, as a field, we had largely ignored the study of normal, healthy, and stable individuals. Including the study of healthy individuals within our scope would help prevention efforts as well as increase the treatment success rate. Clinicians bring out and enhance the patient's strengths, who then face their challenges through mastery and resilience. Treatment plans work to improve the positive psychological traits, including courage, optimism, positive emotions, positive appraisal, positive selective attention, positive internal coping skills, problem-solving skills, humor, resilience, emotional control/regulation skills, and willingness to approach fears. Positive psychiatry also includes direct work on improving family dynamics and social support. Starting from a positive perspective, mental health provider communicates the acceptance, respect, and belief in their patients' capabilities. Rather than discussing anger on parents and their shortcomings for months to years, clinician engages them in the treatment and mediates their conflicts. There has been emerging evidence for the effectiveness of positive psychiatry on mental illness, and still, significant research is needed in this field.

Key Points

- Positive psychiatry broadens the scope of the traditional approach with a high reliance on patient's resourcefulness.
- Positive mental health provider and patient co-investigate and collaboratively solve the problems through shared strengths and expertise.
- Positive psychiatry not only identifies and targets the biological, psychological, and social problems but also recognizes and utilizes strengths in each domain.
- Providers focus on keeping the patient's sense of agency and use more training and teaching to increase their self-efficacy and Personal Mastery.
- Resilience, optimism, and letting go could be learned with practice.
- There is a significant need for more research in the field of positive psychiatry.

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12 Positive Interventions in Schizophrenia and Psychotic Disorders

Narsimha R. Pinninti and Walter Rhoades

Introduction

Positive psychotherapy (PPT) stems from the positive psychology movement recently spearheaded by Seligman and his colleagues [1] and preceded by the work of Nossrat Peseschkian in Germany. In this chapter, PPT will be applied to Seligman's approach, while Peseschkian's approach is described in others (see Chap. 2 for clarification). In PPT the psychological interventions are focused on clients' positive resources, such as positive emotions, personal meanings, and strengths (including existential and spiritual meaning), and utilizing these to help the individuals address their psychopathological symptoms and emotional distress. The mnemonic PERMA captures the essence of PPT. It stands for positive affect, engagement, positive relationship, meaning, and accomplishment. The essence of PPT is building and maintaining a positive relationship through which other interventions are provided [2]. Traditionally, psychotherapies follow the biomedical model in diagnostic evaluation followed by interventions that target most distressing symptoms and deficits. For example, cognitive behavior therapy (CBT) of depression focuses on the negative cognitive triad (negative view of self, the world, and the future), and interventions are designed to change the dysfunctional cognitions [2]. Identifying and increasing self-awareness of strengths is part of CBT, but that is not the central focus, and the majority of the therapy time is spent in developing cognitive conceptualization of the individuals' psychopathology and helping the individual gain this understanding [3]. Improvement takes place through the individual developing more adaptive cognitions, coping skills for physiological reactions, and behavioral patterns [4]. Recent work of CBT for psychosis has focused less on the deficits and more in enhancing recovery through engendering hope and enhancing self-concept [5]. PPT differs from CBT and other therapies in that the majority of time in therapy is spent on unearthing positive strengths and generating positive affect and utilizing these to mitigate psychological distress and reduce symptoms. PPT was initially utilized for depression and subsequently a variety of conditions that are the focus of this book. There has been some work in utilizing PPT for individuals with schizophrenia and other psychotic disorders, and the field in this area is still evolving [6].

Psychosis is the defining feature of schizophrenia spectrum disorders, a common but variable feature of mood and substance use disorders, and a relatively common feature of many

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_12

developmental, acquired, and degenerative neurologic and medical conditions [7]. This chapter covers all psychotic disorders including schizophrenia. Psychosis has traditionally been viewed in categorical terms with psychotic experiences considered essentially pathological. This categorical view of psychotic experiences as pathological contributed to the significant stigma associated with psychotic disorders and has been reported by clients as a barrier to recovery [8]. Recent evidence points to a more dimensional nature of psychosis with a continuum of psychosis ranging from self-reported infrequent psychotic symptoms in the general population to schizotypal traits, to schizotypal personality disorder, and finally to full-blown psychosis resulting in a diagnosable primary psychotic disorder [9]. In this chapter, psychosis is viewed as a spectrum with individuals who are at higher risk for psychosis at low end of the spectrum than those with prodromal symptoms, psychotic experiences, and psychotic disorders including first episode of psychosis, through either a relapsing or chronic course [10]. Positive psychotherapy (PPT) interventions can and should address the entire spectrum and prevent individuals from moving into the deeper end of the spectrum while helping them on their recovery journey.

Traditionally, the mental health system and society viewed schizophrenia as an illness without recovery, and goals of treatment were symptom reduction and preventing hospitalization [11]. However, currently recovery from mental illnesses including psychotic disorders is considered an expectation and reasonable goal by individuals who have recovered, families, mental health systems, and even the governments that ultimately foot the bill for mental health services [12–14]. Along with a change in the expectations for better outcomes, there is also a change in moving away from relying primarily on biological treatments of psychosis. Increasingly psychosocial interventions are given a well-deserved and more important place in their management of psychotic disorders. Current standard of care is that every individual diagnosed with schizophrenia is offered psychotherapy such as cognitive behavior therapy for psychosis (CBTp) along with pharmacotherapy as part of treatment [15]. Clients and their families also prefer psychosocial treatments to biological ones, but the current reality is that psychosocial treatments are labor-intensive and not widely available for individuals suffering from psychosis [16].

One relatively recent shift in the thinking and treatment of individuals with psychosis is paying attention to the experience and perspectives of individuals who have suffered from psychotic disorders including schizophrenia [17]. These individuals who are called "Experts by Experience" describe the importance of a trusting relationship with a professional as critical in their recovery. The relationship provides connection and containment during crises and high degree of autonomy in personal decisions during periods of stability and thereby promotes recovery and personal growth (Saks 2007). Recovery from psychosis is viewed as a journey, and there are several facilitators and barriers on this journey. Many clients describe stigma and discrimination, negative experiences from mental health services, and negative effects of medication as barriers to recovery [8]. The traditional model of psychiatric care termed the "medical model" focuses on the symptoms and deficits of the individual, and individuals with lived experience consider this perspective as a barrier to the recovery journey [18]. Positive psychotherapy for psychosis (PPTp) on the other hand by its focus on the healthy aspect of the individual makes the health-care experience a positive one for the clients and removes one of the current barriers in the recovery journey. The interventions for schizophrenia and psychotic disorders are still evolving and requiring more clinical and empirical fine-tuning. The interventions can be divided into two categories: (a) intensification of positive affect and experiences and (b) enhancing strengths of character. The first step in PPTp is building a positive therapeutic relationship that is mutually respectful, and the power differential in the relationship is minimized. It is important to recognize that there is normally a power differential in the relationship between client and therapist contributing to the stigma that can be addressed by the therapist explicitly expressing

that there are two experts in the room. One is an expert through knowledge and degrees, and another is an expert through lived experience [19]. All decisions about the therapy direction as well as different therapeutic decisions during the course of therapy are made in a collaborative manner with the involvement of the client. It is extremely affirming for the client to have a physician or therapist listen to their intimate experiences without judgment and treat them as an equal partner [20, 21]. Experiencing positive affect in the session and learning to generalize this into real life through skill building helps in mitigating psychotic symptoms, improving affect, and countering the anhedonia that is part of negative symptoms. Positive affect in session is generated by establishing a meaningful therapeutic relationship through techniques such as engaging in casual or water cooler conversation, building on previous positive experience, identifying common interests, validation, and humor of laughing with patients [22]. Internet access provides an opportunity to enhance a client's interests by helping them engage through those interests while in session. For example, an individual who hardly spoke due to negative symptoms was interested in rap music. When the author brought up his favorite song on YouTube in the session and played it, he became energized and rapped with the song and even began dancing. Music became a regular topic of conversation with him in their sessions subsequently, and many times he would just want to talk about music and in the end spent a few minutes on medication. The positive experiences in the sessions are then used as the foundation to identify more activities at home that can generate such an effect. An activity and pleasure schedule can be developed in session, and clients can keep track of their pleasurable activities. The first author has a list of pleasurable activities that he utilizes to jog clients' memory of activities that clients have previously engaged in or can get interested in. The second group of interventions is enhancing the strengths of character of the individual, and this has the effect of instilling hope in dealing with existing challenges and building resilience to deal with future challenges. The first author utilizes a technique to review the client's life story while identifying the strengths and attributes that helped the individual in dealing with the traumas and challenges in life. See the chart enclosed (see Fig. 12.1). Usually after completing the chart, clients get to see the number of

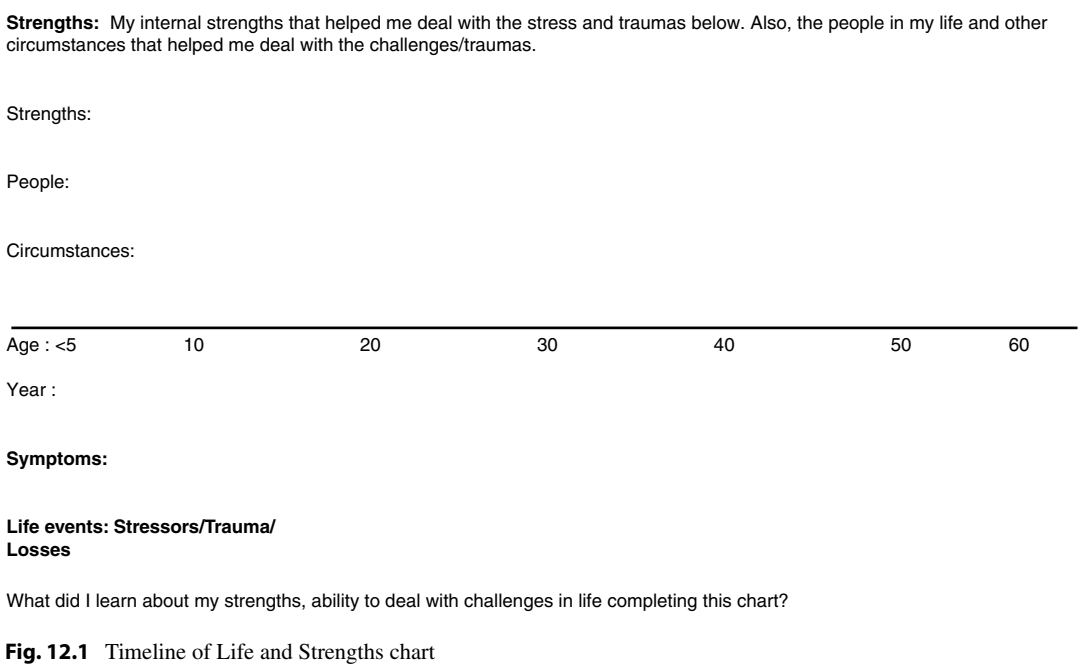


Fig. 12.1 Timeline of Life and Strengths chart

challenges and traumas that they have dealt with in their life and the strengths that they brought to bear in dealing with those challenges. The outcome of this intervention is that clients see themselves as survivors and are more aware of their strengths. In those situations where clients have difficulty identifying their strengths, they can recollect what people who know them best would consider as their strengths or even do a homework assignment to ask what people who know them well think of their strengths. Many times, clients are very surprised to hear the positive things that they hear from their family and other care providers when they specifically ask for feedback.

Empirical Evidence

Positive psychotherapy programs developed so far for persons with schizophrenia vary in content, procedure, and specific goals for interventions, but all are geared toward improving life satisfaction in a lasting and functional way [6]. Overall, the evidence from controlled trials for the efficacy of PPTp is limited and is covered here. PPTp has been provided in individual and group format with more evidence for group format.

Johnson et al. developed a secular version of Buddhist meditation called loving-kindness meditation (LKM) and evaluated it in an open-label study of 18 participants with schizophrenia spectrum disorders and significant negative symptoms. Their conclusions were that the intervention was feasible and associated with decreased negative symptoms and increased positive emotions and psychological recovery [23]. Kim et al. randomized 57 subjects with schizophrenia into a PPTp group and a controlled group who received treatment as usual in a community setting. The PPTp group received ten sessions of individual therapy in 5 weeks. Results showed that interpersonal relations ($F = 11.83$, $p = 0.001$) and resilience ($F = 9.62$, $p = 0.003$) significantly increased in the PPTp group compared to the control group showing the efficacy of PPTp in this study. However, there was no change in positive affect with PPT interventions [24]. More studies on efficacy of individual therapy format for PPTp are needed to recommend this therapeutic modality more widely.

Positive emotions program for schizophrenia (PEPS) is a specific, short, group-based intervention to improve pleasure and motivation in schizophrenia [25]. In a pilot study, 31 subjects with schizophrenia, or schizoaffective disorder, participated in an 8-week PEPS, and the results were statistically significant reductions in the total scores for avolition-apathy and anhedonia-asociality on the Scale for Assessment of Negative Symptoms with moderate effect sizes. Furthermore, there was a statistically significant reduction of depression on the Calgary Depression Scale for Schizophrenia, with a large effect size. Emotional blunting and alogia remained stable and did not improve during the intervention [26]. Riches et al. used an evidence-based theoretical framework to modify 14-session standard PPT into a manualized intervention, called WELLFOCUS PPT, which aims to improve well-being for people with psychosis. The modification was undertaken in four stages, qualitative study, expert consultation, manualization, and stakeholder review, and was informed by systematic review and qualitative data. The resulting WELLFOCUS PPT is a theory-based 11-session manualized group therapy [27]. WELLFOCUS PPT was tested as an 11-week group intervention in a convenience sample of people with psychosis in a single-center randomized controlled trial involving 94 people with psychosis. ANCOVA showed no main effect on well-being according to the primary outcome scale (WEMWBS) but showed significant effects on symptoms ($p = 0.006$, ES = 0.42), depression ($p = 0.03$, ES = 0.38), and well-being according to the Positive Psychotherapy Inventory ($p = 0.02$, ES = 0.30) [28]. Client's feedback about the group experience was positive throughout. Components found helpful included learning to savor experiences, identifying and developing strengths, forgiveness, gratitude, and therapist self-disclosure [29].

Applications

Typically, psychotherapy assumes a dyadic relationship between the therapist and the individual receiving therapy, but the existing reality is that an individual with psychosis has several members of a treatment team working with them at any point in time. A comprehensive way of applying PPTp principles and practices is to include all the members of the treatment team in this endeavor, and together the treatment team can provide these interventions. Where possible it can be helpful to bring the family and social connections of the individual into the orbit of the therapeutic circle so they are working hand and glove with the treatment team toward the individual's recovery. This concept has been utilized in a method called open dialogue in Finland, and they report significant reductions in the use of medication, reduced psychiatric symptoms, and most individuals recovering their life roles [30].

Vignette

HP is a 51-year-old African-American man with a diagnosis of schizoaffective disorder and posttraumatic stress disorder going back to his early 20s. He was enrolled in an assertive community treatment team due to a history of medication nonadherence and repetitive verbal as well as physical aggression in other settings such as a partial hospitalization program. In his last partial hospitalization, he flipped a table causing a fracture to a staff member and was put on probation with the possibility of doing jail time for repeat aggression. He was on depot haloperidol 150 mg IM every 4 weeks and sertraline 50 mg AM to help with persecutory delusions, ideas of reference, depressed mood, and angry outbursts. First author (NP) did medication monitoring at four weekly intervals, while the rest of ACT team members saw him on a regular basis to help him with his instrumental, vocational, and relational needs. He was given the option and choice to be seen earlier than 4 weeks if he so chose. The symptoms that bothered the client were:

(a) Ideas of reference that people were judging him.
(b) Paranoia that people are against him.
(c) Depressed mood and beliefs that he is worthless.

These beliefs when triggered would lead to verbal aggression including threats to harm staff. His developmental history was relevant for enduring severe physical and emotional abuse by his mother that lasted most of his childhood, his two brothers ended up in jail, and the daughter he had through a relationship was given in adoption without his knowledge, and he did not know her whereabouts. In essence the ACT team was his only professional and personal support system.

Clinical Issues and Interventions

The main issue with HP was repeated angry outburst and sometimes being depressed and hopeless with suicidal thoughts. We saw his anger and depression as being linked with stress pushing him into one or other, and he could easily move from anger to becoming very depressed and hopeless. Positive interventions were utilized when he presented with these symptoms. The first focus was on building a positive relationship with clients. This was done by allowing him to define his own short- and long-term goals, validating his emotional reactions to various situations, and normalizing the emotions but at the same time identifying his behaviors such as verbal outbursts as barriers to his life goals. The relationship was strengthened when the focus in some sessions was on identifying the client's positive values, attributes, and decisions and bringing them to the attention of the client. A typical session would start with the client reporting his problems. Then therapist would validate clients' thoughts and emotions, and together they would review:

(a) Recent positive things that the client did.
(b) Positive decisions that the client made over a period of time.

(c) Clients' positive attributes and values that have been discussed and agreed on before.
(d) The adversities that client faced and skills he used to overcome them.

About 80–85% of the session would be spent in these interventions, and typically that would bring about a change in his affect from anger and irritability to depression at first and then of relaxation and cheerful affect. The therapist would then point out to the client the change in his affect and how this affect makes the client a fun person to be with. Most of the time, these positive interventions would be adequate for the client to feel much better about himself and go back with increased self-assurance and with increased motivation to continue his treatment. At other times, he would also ask for some adjustment in medication. All medication decisions were made collaboratively with complete involvement of the client, and unless there was a real reason not to, the psychiatrist would go along with the client's request. This was with explicit understanding that the client would inform us in case the change made him worse. On several occasions, he would reduce the dose of medication and a week later call to go back on a higher dose. This way of allowing him the opportunity to make trial-and-error medication decisions made him feel empowered and contributed to improved self-worth. On several occasions, the client left the meeting saying "I am better when I come and talk to you" or "You make me feel good or I do not know what I would do without the ACT team. You are the only family I have got." While the first author met with the client infrequently, the rest of the team saw him at least once or twice a week. The first author described his interventions and what was effective to the entire team, and they adopted the same approach of positive interventions to help him overcome his symptoms. The above is a snapshot of some positive psychotherapy interventions, and in addition he received a variety of services that included vocational housing support, budgeting, vocational counseling, and relationship counseling by different ACT team members [31]. While enrolled in ACT, he was able to make progress in every aspect of this life. He completed probation without any problems and never had another episode of physical aggression. Thus, he avoided incarceration and maintained his own apartment, working part-time. He also entered into a relationship and maintained a relationship for a period of 8 years.

Positive Interventions in a Team-Based Approach to Address Physical Health Crisis

AJ is a 36-year-old African-American man with diagnosis of schizophrenia, moderate cannabis use disorder, and medical problems including obesity, type 2 diabetes mellitus, and hypertension. He lived in a supported housing facility with case management and was scheduled to attend a partial hospitalization program three times a week. His main issues were partial medication adherence, paranoid thinking, and thought disorder of mild degree leading to nonadherence with medical care. During a routine med visit, he was found to have significant hypertension for which he was sent to the emergency room and was hospitalized. He was diagnosed with chronic renal failure and recommended dialysis. This was a crisis for him, and he reacted to this crisis in physical health by becoming paranoid about the doctors, refused to cooperate with any investigations, and wanted to sign against medical advice (AMA). At this time, his regular psychiatrist coordinated care with a psychiatric consultant on the medical floor to come up with a plan that AJ as well as the medical team was comfortable with. AJ was willing to meet the team half way and get some investigations such as ultrasound in the hospital, but he would not undergo a shunt for dialysis. He agreed to follow up with his PCP soon after discharge. This plan was also acceptable to the nephrologist, and he was discharged with this follow-up plan. When he returned to the partial hospitalization program, the psychiatrist coordinated interventions with his case manager in the supportive housing program and counselor at the partial program to strengthen the relationship with the team. While the case manager worked with the client in the community and his

residential settings, the psychiatrist and the counselor worked with him in the partial hospitalization program. Within the framework of strengthened relationship, the team focused on identifying the adversities that AJ has faced before and the adaptive ways in which he coped with them. The team also identified the strengths he has exhibited both here in the program and the group home, as well as his emotional experience while in the program. When different team members were able to shine light on his strengths in different situations, there was visible change in his affect from one of anxiety to being more relaxed, positive, and hopeful about his future. He was able to acknowledge this positive change and did verbalize that speaking to and being with team members was positive for him. Every small decision he made such as getting labs done, going to PCP, filling his scripts in pharmacy, and coming to program regularly was highlighted, and these were reframed as facing him showing courage to face normal and expected fears. These actions were also highlighted as helping him work toward his long-term goals. With these interventions done, AJ agreed to get a shunt placed in his arm for dialysis as the first step and followed it with starting dialysis. He has so far kept up with his dialysis schedule 3 days a week while coming to the program 2 days a week, and he no longer has any fears about the dialysis. His labs that were reaching potentially critical levels are now improved. He reports feeling better physically and reports that color returned to his skin. His mental state showed improvement in that the thought disorder and paranoia disappeared and anxiety came down. AJ continues to be medication and treatment adherent at this time.

Both these vignettes show the use of PPTp interventions for individuals with psychosis in naturalistic ACT teams and partial hospitalization settings. While it is important to have the efficacy of PPT demonstrated in controlled trials in one-to-one or group sessions, most individuals are treated in community health-care settings such as partial hospitalization. In these settings, it is difficult to have the luxury of resources where individual clients can be seen for 8–15 sessions on a weekly basis. What is needed is to fit the interventions into the normal workflow of the existing treatment teams so that more people can receive these interventions. These two case vignettes show that PPTp interventions can be incorporated into the routine workflow and by involving the entire team in providing these interventions, meaningful outcomes can be achieved.

Areas That Need More Research

PPTp belongs to an emerging generation of therapies and is a promising therapy approach for individuals with psychotic disorders. However, there are several areas that need further research and clarification. First is that there is limited research evidence for PPTp delivered in an individual format, and more research is needed with rigorous methodology. For the group therapy format, there is more evidence, but there is not one standardized protocol that most groups of researchers follow. As a result, it is difficult to compare different studies that use different interventions. What is required is the scientific community being able to coalesce around a single protocol which can then be tested in multicenter trials to demonstrate efficacy. Second issue with PPTp is an overlap between CBT for psychosis, mindfulness-based CBT for psychosis, and PPTp. Positive psychotherapy for psychosis interventions have to be delineated clearly so that same or similar therapies are not studied under different names. The third issue is that we do not have adequate effectiveness data. We need more studies on the effectiveness of PPTp interventions in real-world situations as the success of this therapy modality will ultimately be determined by feasibility and effectiveness in real-world situations. A fourth issue is that staff from different disciplines such as psychiatrists, advanced practice nurses, registered nurses, therapists, counselors, case managers, and peers are involved in treating individuals with psychosis and future studies need to be done to show which of these staff can best utilize PPTp intervention. Similar studies have been done in cognitive behavior therapy for psychosis [32, 33]. Families

of individuals with psychosis face enormous burden, and future research on PPTp can evaluate whether family members can be taught interventions that help them interact with their loved ones effectively and if they can utilize these techniques to reduce their own burden.

Summary

To summarize, psychotic disorders are more common than we recognize, and many individuals do not discuss their symptoms due to significant stigma. PPT is very well suited to address psychotic symptoms and improve functioning through relationship building and engagement. PPT interventions are best viewed as central to a dyadic relationship between staff member and the client. This is complimented by client's social and professional circle being made part of the therapy process through education and skill building so that the experience of clients in dealing with the most meaningful relationships in their lives generates positive affect and helps them gain meaning and thereby enhances quality of life.

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Positive Psychotherapy and Eating Disorders

13

Maksim Chekmarev

Introduction

There are two main eating disorders in ICD-10: anorexia (AN) and bulimia (BN). There are marked variations in prevalence across cultures. Several diagnostic manuals reasonably add binge-eating disorder (BED) to this group. These presentations are described in the sixth chapter of ICD-10 F5 "Behavioral syndromes associated with physiological disorders and physical factors." This category shows the syndrome-oriented and biologically oriented ideology of a large number of psychiatrists instead of focus on the cause approach. Meanwhile, comparing the national recommendations for the treatment of eating disorders, the importance of psychotherapeutic and socio-therapeutic approaches and the relatively small contribution of pharmacotherapy are noticeable [1]. The empirical nature of the recommendations strengthens the idea that the leading causal factors of these disorders are psychogenic, and, therefore, different modalities of psychotherapy are needed to offer a clinical understanding of eating disorders. We will try to outline a range of approaches to overcoming eating disorders from the point of view of positive psychotherapy (PPT after Peseschkian).

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General Description of Eating Disorders

Eating disorders can be understood from three main perspectives: appetite; attitude toward food; and episodic nature [2].

The first is appetite, or the desire to eat. On the one hand, there is anorexia, with an inhibited desire to eat. On the other, bulimia and BED are linked with normal appetite and can be periodically intensified.

The second is attitude toward food. Is it good or bad, dangerous and painful, or useful and enjoyable? In anorexia, food is an enemy; in severe forms, the rejection of food is seen. Patients dream about freedom from the need to eat; they can accept eating only for a basic level of living. For BED sufferers, food is a pleasure, but within the episode of compulsive eating, pleasure can be lost. Complaints from patients are focused not on the food but on the loss of self-control, the increase in body weight, and the feeling of shame. Bulimia has an intermediate position. Appetite is normal, as is the capacity to achieve pleasure from eating, but food is perceived as something bad, especially when it has already been eaten. Patients with BED are able to feel pleasure during eating, but begin to feel aversion to the state of satiety and filling of a stomach.

The third perspective is the episodic versus chronic nature of each disorder. Bulimia and

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_13

BED are cyclic disorders. The usual style of eating is normal, but periodically, episodes of a disorder occur. Anorexia entails more chronic inhibition of the appetite.

Definition of Terms and Particular Description of Eating Disorders

Anorexia nervosa is a psychic disorder with a distorted perception of the body as esthetically unpleasant, imperfect, even disgusting. This distorted perception is connected with inadequate feelings with regard to weight and other parameters of the body. Patients use extreme methods to attain the perfect (from their point of view) body: refusal of food or severe limitation of it, increase in physical activity, and use of drugs (diuretics, laxatives, lipase inhibitors). These practices lead to significant weight reduction and its consequences may include oligo- or amenorrhea, myocardial dystrophy, and/or decrease in blood pressure. High demands on oneself extend not only to the body, but also to other spheres of life—work and study, personal relationships, intellectual and spiritual development. Patients may achieve a lot, but are not satisfied with it, there is always the idea that efforts are insufficient and achievements are ordinary. It is very difficult to recognize yourself as being good enough because of the fear of ceasing to develop; experiencing shame and guilt is typical and it is also linked to feeling that anorexia causes suffering to loved ones. On the other hand, the patient's perception of her/himself as underdeveloped may be associated with rejection of the role of a typical "adult." A similar pattern takes place in bulimic disorders. This is finely described in Ray Bradbury's story "Once upon a time in the days of eternal spring." The protagonist of is a boy who considers that food is a poison because it promotes adulthood. The character eventually overcomes his conflict through love. Several patients with eating disorders long for love, but consider themselves unworthy. Refusal or restriction of food is often associated with greater ethical considerations: the idea that there are countries in which children are starving; the rejection of violence associated with the use of meat and another animal products; the benefits of asceticism, which can be perceived as a method of self-improvement; and a way of purifying through punishment. Ideological motives can make the patient unready for cooperation during a long course of psychotherapy, especially adolescents, for whom solving the problem of nutrition means fulfilling the requirements of parents or the adult world.

Bulimia is an eating disorder characterized by cyclicity: periodically, there is a difficulty in controlling the desire to eat, the patient eats a lot of food, often high-calorie food, rich in fats and carbohydrates, and then because of psychological and physical discomfort, induces vomiting to empty the stomach. The described eating behavior is observed in patients with high demands for success in all spheres of life, with a very narrow path to self-acceptance that the patient allows only in the case of extraordinary achievements. Bouts of overeating with the consequent stomach-emptying are usually linked to a complex range of feelings, in which need and pleasure are mixed with a sense of guilt, fear of losing control, and shame.

Persons with bulimia may resort to the use of medications to control weight, including laxatives, emetics, diuretics, and lipase blockers. Vomiting may be induced mechanically as well. In that process, patients sometimes intentionally or unintentionally inflict damage to the mucous membrane of the pharynx and oral cavity.

It should be noted that the vast majority of patients are well adapted at work or in school, and other people often perceive them as being highly functional to the point of arrogance.

However, failures in love and friendship, suffering in a close relationship, are among the typical reasons that cause the greatest suffering to patients. They are desperately trying to satisfy their hunger for love, affection, and meaning.

The function and meaning of the symptoms in bulimia can be described as follows. On the one hand, the symptom complex includes quenching physical and symbolic psychological hunger, as

an attempt to fill oneself and one's life, to find joy and acceptance. On the other hand, encouragement is often followed by punishment in the form of physical discomfort and vomiting. As a consequence of this, a feeling of control over the patient's life is achieved. The patient feels the need for love, attention, pleasure, and meaningfulness, but does not believe that he, or she, is worthy of them as a result of high achievements, the bar of which rises higher and higher. Overeating helps to achieve temporary satisfaction, and inducing vomiting helps to restore the status quo, getting rid of guilt and shame. Behaviors associated with bulimia allow the temporary satisfaction of the needs that the patient unconsciously refuses to meet.

Some authors speak of a bulimic form of anorexia, or bulimorexia, characterized by the dominance of the theme of refusal of food, the perception of food as malicious, and the formation of a developed system of self-restraint. In this presentation, excessive anxiety about food may cause spontaneous nausea and vomiting. The loss of body weight becomes the most noticeable symptom, and among the psychological features, there is an excessive need for self-requirements and maximalism. Self-assessment is often built on the principle of "or–or": "I am either successful or worthless." A very strong theme of guilt, lack of acceptance, and thrift bring this form closer to anorexia.

Binge-eating disorder is an eating disorder with attacks of increased appetite, followed by abundant absorption of food. The patient finds these attacks difficult to control.

The pattern of relationships between the person and the body can be different. Dissatisfaction with their appearance, being overweight, and self-condemnation for overeating are typical, but the patient usually does not take active consistent actions to address these problems. Food is perceived as a pleasure for which you have to pay by being overweight and having related health problems. Very often, food becomes a way of relieving anxiety, filling empty time, and/or coping with boredom. Eating is a fast and reliable way of feeling satisfaction, of rewarding yourself. The patient often does not receive expected rewards in various spheres of life, for example, he or she does not feel appreciated at work, there is no activity on the part of the partner in the sexual sphere, and he or she does not feel interest from other people in friendships. Sometimes, overeating is associated with the collapse of expectations and the despair of getting, or feeling, anything.

The choice of food reflects this attitude. Choices of enjoyable and useful products associated, in the culture of the patient, with good social status, indicate a fairly intact self-perception and capacity to care about the self. Other times, patients eat harmful food, which may indicate a sense of despair and problems with the perception of their own personality.

In either case, the patient goes through a phase of stress accumulation associated with a specific stress, and then, when the stress level exceeds the threshold, food excess follows.

The function and meaning of the behavior of compulsive overeating can be characterized as the ability to achieve rapid satisfaction of needs under conditions of disbelief in the ability to receive acceptance, pleasure, and attention under the usual circumstances of human relationships.

As thus understood, eating disorders are not directly related to nutrition. They are associated instead with the symbolic meaning of food in our lives [3, 4]. Thus, the unique pattern of symptoms of each disorder becomes a support for us in understanding the meaning and function of what is happening with the client. This understanding helps the client and the process of psychotherapy to get away from the approach of overcoming the disorder as a struggle. We have already found out, characterizing the three perspectives of clinical manifestations, that hostility to oneself overwhelms the inner world of the client, and should be overcome. It is appropriate to assume that such a hostile perception creates a vicious circle of feelings, supporting clinical manifestations and eventually strengthening them. What is the center of these experiences? What position of the therapist is able to create a "lever of action" to make changes necessary to overcome the disorder in the life of the client?

General Psychotherapeutic Considerations for Eating Disorders

The position of the therapist, which helps to find a foothold for the patient and start effective internal work, is directly related to the ability to rely on the positive concept of the person, which indicates the presence in the client of the two basic capacities: to love and to know [5, 6] (see Chap. 2).

When working with disorders affecting physiological needs, therapy relies on self-knowledge through the perception of oneself as the field of study. This means the need to move in the client's self-perception from reflex to the reflexivity, from reaction to introspection.

This reflex, in fact, is one of the key features in the vicious circle of eating disorders. It is associated with an immediate and often unconscious response to a stimulus. There are at least two circuits of such reflexes: to external circumstances that provoke an exacerbation of the symptom, and to the symptom itself, which becomes the reason for the reaction. For example, the client feels an irresistible urge to overeat before an exam, which helps to drown out the anxiety, but after an episode of overeating, s/he experiences an acute sense of shame that s/he was not able to restrain him/herself. This shame can also be associated with the fear of getting fat, becoming unattractive, feeling desperate that the condition is out of control again and again, and will never get better. Emotional chains then branch out, pointing to the frustration of significant needs: on the one hand, in self-confidence, the ability to cope with the exam or other difficulties, and, on the other hand, in acceptance and self-love, which are impossible for the client outside of the conditions of success and control of their physical form. However, in the process of activation of the symptom, these emotional processes are not visible, become conscious only post factum, and, sometimes, are not realized at all. The reflex pathway is the path of action, which is carried out in line with the concepts already available to the client. Strictly speaking, the behavior itself is a product of the reflex pathway, which is a reaction to the current problem, but does not offer a resolution [7]. The theory of PPT connects it with the processing of the actual conflict, which, owing to the severity and significance of the symptom itself, is in the shadows and is not realized. The client's focus shifts to his/her reactions, s/he begins to strive to suppress them, failing time after time, because the main source of tension is not eliminated, and periodically capitulates before the symptom, which causes aggravation of the experiencing of the problem and the occurrence of secondary painful reactions and disorders, makes him/her look for extreme and unnatural ways to solve the problem, such as excessive restriction of nutrition to starvation, exhausting exercise, surgical interventions on the gastrointestinal tract or removal of adipose tissue. The reflex pathway creates a feeling of powerlessness, because the client perceives him/herself as an object of suffering. S/he perceives the disorder as inseparable from him/herself, despite all attempts to fight. Thus, all the ways of fighting against the disorder are a struggle in themselves.

Moving from reflex to reflexive activates our ability to know ourselves. The very ability to reflect that is inherent in human beings from birth is a basic element of self-knowledge, representing an opportunity for consciousness to use its own psyche as an object of knowledge, becoming a subject, i.e., an active personality [8]. This position creates a synergy with the psychotherapist, who is able to catalyze the process of self-knowledge, being an external participant in human relations with him/herself. A reflexive position is not healing itself, but it is a necessary condition for healing to begin. Its main essence lies in the ability to experience impulses from the position of the observer, paying attention to the processes behind them. The client moves from identifying him/herself with the problem to considering it as part of his/her present and past life history, from the problem history to the possibility of perceiving it as real and multifaceted, in accordance with the PPT model. A reflexive process starts to take place between the trigger and the reaction. It needs a gradual expansion to enable the client to create a thinner and clearer picture of his/her inner world. However, for the

client, this observation can be unbearable and painful if the reflexive process is not connected with the acceptance–actualization of the basic capacity to love.

The basic capacity to love in the process of differentiation becomes the source of an expansion of our capabilities to build relationships. The distortion of the image of one's own body and of the perception of physiological needs indicates a lack of acceptance, alienation of a part of oneself, the inability to treat this part with emotional warmth and acceptance. Most eating disorders are characterized by a feeling of insufficiency and wrongness of the desire to get rid of or drown out these "wrong" parts. The client comes to therapy at times, flooded with these self-perceptions. Without understanding the reasons for such emotional pain, s/he refuses to accept something inside her/himself, and therefore feels an irresistible need to do something, which is implemented in the field of eating behavior. Impatience provokes the patient to hurry, to seek advice and direct recommendations from the therapist, to be more in the sphere of "doing" rather than "being." It is easy to see that we have met with the second part of the vicious circle, leading us away from the very essence of the client's experiences. But we cannot go further if the client is not there. We cannot accept what the client her/himself has not yet accepted. It is logical that the therapist himself becomes a donor and an example of acceptance. We start by accepting the client's rejection of himself. This acceptance potentiates the work in the reflective position; it would validate those experiences that the client sees in his inner world. True love is far away, but the starting goal is to develop the forces of "being" with problematic experiences. Therefore, we gradually get the opportunity to touch the root causes of eating disorders. The client's ability to meet emotional pain and see it as part of the internal and external context is expanding [9].

Thus, by implementing the presence and development of basic abilities in the client's problem situation, we are able to see the experiences behind the symptom and not reject them too quickly. We stay with the client in the field of his inner world to clarify the conflicts behind the disorder and develop relevant abilities that can become a resource in the process of overcoming. Now, the client with the help of a psychotherapist is able to dive into the safe space of therapy in communication with a specialist to the level of critical experiences, which allows this process to be life-changing [10].

The general principles of the approach to the treatment of eating disorders focus on three areas, corresponding to the leading concepts of PPT:

1. Balance: an integrated approach to therapy affecting all spheres of human life, the analysis and harmonization of all spheres of life.
2. Conflicts: the study of conflict dynamics and psychogenic causes of the disorder at all levels—actual, key, basic, and internal conflicts. The work is aimed at understanding the causes of the disorder.
3. Capabilities: includes diagnostics of the actual capabilities of the client, understanding of the opportunities for development of those capabilities, which are involved in the formation of problem situations. The assessment of resources and deficits of the client, the updating of resources, and the overcoming of deficits through development.

The principle of balance calls for the need for an integrated approach toward eating disorders that requires the participation of a team of mental and somatic health specialists at the different stages of treatment [11].

The Importance of the Interprofessional Team

Most of the national guidelines for the treatment of eating disorders are based on an integrated approach, including and sometimes centered around psychotherapy.

Despite our opinion on the predominantly psychogenic origin of eating disorders, considering the idea of balance, we believe it is important to coordinate different professionals, which will make the team most effective. This also reflects the holistic approach of PPT. Therapy affecting

all spheres of human life creates a special psychotherapeutic environment that becomes a support for the client during the entire period of work with him or her.

A few clarifications for the following sections: it is assumed that the coordinator of the therapeutic process is a psychotherapist, working with a general practitioner, and, if necessary, a gastroenterologist, endocrinologist, and nutritionist. Group therapy is useful and it is highly desirable to gain the support of the family and, if the client is religious, of a community of believers.

During psychotherapy, we should provide changes in each sphere of patient's life: body, activities, contacts, and meaning (for the Balance Model, see Chap. 2).

Sphere of Body

- Exploration of the experience of the disorder, the somatic consequences of the disorder, especially with its long-term persistence (damage to the teeth and oral cavity, esophagus, stomach, endocrine glands, nervous system, etc.)
- Assessment of the main parameters of the physiological sphere; if necessary, the implementation of the necessary somatic treatment
- Investigation of the nutrition status; its gradual correction

Sphere of Activities/Achievements

- Activities to improve the client's knowledge about her/his disorder
- Assessment of the impact of the disorder on the educational and work activities of the client, as well as the impact of his/her educational and professional activity on the disorder itself

Sphere of Contacts

- Assessment of the influence of family and social environment on food behavior, the course of the disorder, and the psychological state of the client; the involvement of the social environment in the care system, if necessary
- Participation in a support group or the opportunity to meet people who have overcome an eating disorder

Sphere of Meaning/Future

- Clarifying the client's ideas about his/her goals in therapy. How does s/he see his/her optimal state? Discussion of this image, maintenance of its implementation
- Construct a phased plan for life change
- Enhance the understanding of the impact of the client's value system on his/her condition
- Gain support from the spiritual community

The general principles of psychotherapy also affect the role model, the conceptual field of the client, focusing on the past, present, and future, which means the importance of studying the client's position in three directions [12]:

- *What was the attitude toward food in my parents' family?*
- *How do I feel about food now?*
- *How do I see a healthy attitude to food in the future?*

Particular Characteristics of a Positive Approach to the Treatment of Eating Disorders

Nossrat Peseschkian (2016) classified eating disorders as psychosomatic disorders [4]. This fact means that the very nature of the disorders is linked to manifestations of psychological dysfunction through the body. In the process of psychotherapy, we should use actual symptoms, and current conflicts, and move forward to address the internal conflict. The positive interpretation of the symptom helps to clear up the actual conflict behind the symptom.

The path from the actual conflict to the basic conflict—from the external behavior to the psychological roots—is carried out through the search for frustrated need or frustrated value behind the actual conflict [13]. The framing of an internal conflict is based on searching for the concept that confronts another concept that has become dysfunctional in a basic conflict.

In the case of psychosomatic disorder, the key conflict resolution usually tends toward politeness. But sometimes we can see an exception that occurs after long attempts to handle psychological stress in the internal space, when the patient finally snaps and briefly shows powerful aggressiveness.

In the following, we describe the typical options for each eating disorder.

Anorexia

From Symptom to Conflict Through Positive Interpretations

Anorexia is the ability to achieve goals by extreme methods, reaching the ideal by will and through excessive effort. Realization of the goal is usually tied to the capability for achievement that supports a person's sacrifice and self-restraint.

Anorexia is often accompanied by feelings of failure, lack of effort, and it often arises and intensifies during life periods when external evaluation is especially important, such as school exams, development, or end of a romantic relationship. Anorexia is a disorder that other people notice; it is visible. And it becomes a way to show other people one's zeal, to mark boundaries and values.

From Actual Conflict to Needs and Values

Showing extreme zeal for the goal, from the one side a person seeks love but from the other—and it is often crucial—a person needs trust and expects others to find him/her worthy enough to rely on, to recognize his or her autonomy. Thus, the basic conflict is expressed by attempts to gain love and trust by showing extreme zeal in achieving the ideal.

From External to Internal Conflict

Usually the need for love and trust faces the inability to speak of desires and values openly. Openness is associated with the risk of losing relationships with those who are important. Instead of openness, a person chooses politeness and obedience, creating a dual reaction. It means that the need for autonomy and hope of relying on someone's opinion to obtain security go hand in hand.

Bulimia

From Symptom to Conflict: Positive Interpretation of the Symptom

Bulimia can be described as an attempt to obtain the pleasure and love a person desires, but believes themselves to be unworthy of. Symptoms include encouragement and punishment, love and shame. Usually, the appearance of a patient with bulimia does not change; in the case of this disorder, suffering is applied to a person him/herself. He or she does not feel good enough and vicious; the person chooses extreme measures to curb the effects of his/her cravings.

From Actual Conflict to Needs and Values

In a conflict, and in the case of anorexia, there is a certain feeling that achievements are not enough; a person hopes to feel loved by others and to allow him/herself to be loved.

But at the same time, he/she is afraid to lose relationships with others, afraid that they will turn their backs if he/she is going to be less perfect. Thus, a person seeks love and contact by showing zeal.

From a Basic Conflict to an Internal One

The basic conflict in bulimia is linked to high standards, not only for oneself but also for others.

Justice is a more developed ability than love.

A person, therefore, is so desperate for communication and other people's attention, but at the same time rejects many of the people around because they are not well-suited for communication. This increases fear and loneliness.

A person wants to be loved, but in fact is ready to accept love from a very limited number of people.

Bulimia is often characterized by a paradoxical solution to a key conflict. Although showing a lot of politeness to significant people, customers can be very open and even aggressive toward those who are not considered good and successful enough.

Binge-Eating Disorder

From Symptom to Conflict: Positive Interpretation of the Symptom

The symbolic meaning of food may be different; thus, the subtle meaning of a symptom may also differ, but in general food brings pleasure and takes the edge off.

Tension and anxiety are reactions to the unreliability of people and the world around. The need for pleasure is linked to the desire for approval.

Thus, binge eating is a way of pleasing oneself and of alleviating concern under insecure conditions and lack of self-confidence.

The effects of overeating are visible to people around, and the impact usually make a person less attractive to others. There is an opportunity to verify that other people will stick around even for unattractive person, will support him/her, will consider him/her good enough and sexy.

From Actual to Basic Conflict

Checking the reliability of relationships helps to find out if there are reasons to worry.

In the process of exercising due diligence, a person is fueling self-confidence.

Binge eating helps to fix an injustice, to evaluate oneself if feeling underestimated.

From Basic to Internal Conflict

The contradictory nature of overeating causes the uneffectiveness of just improving eating habits and increasing physical activity.

A person is not seeking confidence in an attractive body; he/she tries to establish self-confidence, to be important to others.

The frequency of overeating episodes is reduced in a supportive, careful, and safe environment.

But the client himself usually does not show a strong resolve to set boundaries and release his sphere of contacts from difficult people.

He is afraid to stay alone as a result of choosing with whom to communicate and whom not. At the same time, weight normalization means returning to a competitive environment in which physical attraction is very important, but clients usually do not feel confident under conditions of competition.

Important Elements in the Psychotherapeutic Treatment of Eating Disorders

We conducted a survey of clients who had undergone treatment for eating disorders with the help of specialists who practice PPT. Clients were asked to answer exactly what they found to be most valuable in the psychotherapy.

Here is a list of items that were noted by patients with each form of eating disorder.

Anorexia

1. Less fixation on food, especially reducing the degree of nutrition monitoring by relatives. This reduces anxiety and leaves individuals with a feeling that they are trusted.
2. The chance to share your anxieties and beliefs about food and nutrition with a psychotherapist, and the chance to discuss it in a climate of acceptance and security.
3. Better understanding of links between one's psychological state and the current state of anorexia.
4. Growing ability to cope with life problems through the concept of moderation.
5. The chance to see oneself differently through the eyes of a therapist—with less self-criticism, greater acceptance, and through the prism of resources but not shortcomings and weaknesses.
6. Expanding the idea of attractiveness

Binge-Eating Disorder

1. Therapist's faith in the possibility of healing. Accepting even after failures but without connivance.
2. Expanding the variety of life pleasures, finding the ability to see joy and value in things that have not been perceived in such way. Growth of the ability to value oneself.
3. Assignments that the therapist gives and that help to organize work on oneself.
4. Shaping a more optimistic attitude to one's life, understanding how to cope with anxiety.
5. Expanding ideas of attractiveness.

Bulimia

1. The realization that bulimia is only an effect, but the reasons are distorted self-perception and low self-esteem.
2. Chance to accept oneself, one's current weight and figure as they are.
3. Improvement in self-esteem.
4. Escaping from the "excellent student syndrome."
5. Normalization of nutrition as a result of doing the psychotherapist's homework. Doing the right homework gave a chance to feel that one can live without vomiting.
6. Using not food, but psychological means to manage stress.
7. It is possible to more easily take an episodic weight gain because it is a natural process that depends on the menstrual cycle, the amount of water in the body, and other factors.
8. The realization that it is possible and it is necessary to love oneself not because of a good figure, but in principle because of who he/she is.

Summarizing the main points of the patients' feedback, we can determine a plan for psychotherapy as meeting three main challenges—emotional, rational, and behavioral—and this allows us to use the theory of three stages of interaction of PPT as a framework for the plan [5].

In psychotherapy, we go through three stages of interaction with the client step by step, and these are repeated in each session as far as in the general course of psychotherapy, and this reflects the dialectical principle of development as a spiral.

First Phase: Attachment

The client is dominated by emotional needs: he/she has a lack of acceptance, trust, confidence, he/she is faced with feelings of guilt and shame, fear of not adapting to all the expectations. Secondary complications are associated with the deterioration of relations with the people who often demonstrate misunderstanding, if they are aware of client's disorder, who turn away from him/her or build a system of excessive control.

The patient wishes to gain people's acceptance but faces rejection. The second part of the complications is usually linked to the area of the body and health problems. At the moment, it is important for the client to get a new experience of relations with the therapist, the experience of being understood and accepted, to find himself in the atmosphere of empathic active listening, to feel an interest in his personality. This allows the client to touch his/her experience with greater courage, to verbalize his/her own feelings in the safe space of therapy, increasingly aware of the fact that psychological problems are indeed the reason for eating disorders.

The focus is shifting from disorder to a personality; we begin to analyze a client's conflicts instead of his/her struggle concept, we look for resources and find out the vision. Work at the first stage is crucial; it gives those customers who are focused on secondary capabilities the opportunity to find themselves under conditions where actual primary capabilities play a major role.

It is difficult for a client to accept that love, time, trust, and confidence in his abilities can be acquired without extraordinary efforts, but step by step, he/she feels more comfortable, resistance decreases, and the client begins to admit he/she has the right to satisfy emotional needs simply because they are the basis of any close relationship.

The development of this feeling usually coincides with the possibility of at least temporarily avoiding the existing pattern of disorder; the client is ready to expand his repertoire of ways of coping with stress.

Second Phase: Differentiation

Reflects the work in line with the rational needs of the client.

First of all, we can talk about the progressive development of his/her ability to see the mechanism that triggers the disorder. The client learns to notice causality between life circumstances and nutrition problems. He/she expands his/her vocabulary to describe his/her own inner world, to verbalize emotional needs. An important field to explore in therapy is a set of a client's ideas of physicality, attractiveness, their origin, and development.

This is the area where a person meets a cultural environment, and where it is very important to obtain an expanded vision of oneself in keeping with transculturality, to understand the non-absolute nature of standards of attractiveness, their artificiality.

We explore a wider framework of concepts: the client's view of her/himself, which forms his/her "I-concept" and the manner of his/her self-perception, noting contradictory and narrow concepts. The differentiation phase helps one to start questioning one's usual beliefs and ways of life, giving the client an active role and running self-help mechanisms.

Third Phase: Separation

Extends ways of coping with problems and helps to satisfy the client's behavioral needs. At the same time, the client him/herself turns into an active player, who creates both strategy and tactics to overcome his/her problems hand in hand with the therapist. It is very important to develop openness, which increases the client's ability to resolve interpersonal difficulties, as well as the ability to see his/her own resources. The client knowing the causes and functions of his/her disorder can begin the search for those mechanisms that will help him to achieve the same goals in a healthy way. Clients also note the importance of homework that the therapist gives at this stage: they organize work on themselves, become a source of a new vision and new ways of responding in the life of the client. Homework is individual, but often follows the principle formulated by Nossrat Peseschkian: "If you want something you've never had, you've got to do something you've never done"; it helps to expand the system of goals and values of the client.

A good, stimulating way to correct self-perception is to keep a diary of the client during the psychotherapy process. It is important to note that this is a psychotherapeutic diary, centered on the inner world; it is a counterweight to fixation on the outside world and physicality, helping to achieve a balance between attention to the body and attention to the inner life.

Additional Comment

Most of the national recommendations are unanimous on the importance of involving the client's immediate environment in therapy. In the case of working with children and adolescents, family psychotherapy is necessary. The immediate environment creates a context in which there is a patient with his/her disorder; thus, the elimination of micro-traumatic circumstances in this context helps to overcome the disorder. It is important to make the family a resource, not an enemy.

The average course of psychotherapy lasts for about 1.5 years and at least 50 sessions. The prognosis depends on the associated conditions, the duration of the course, the resources of the client's personality, and the possibility of creating an interprofessional team of specialists [14]. Eating disorders may recur, although the course of psychotherapy in the case of relapse is usually much shorter. In addition to individual therapy, it is reasonable to consider adding group psychotherapy, usually close to the end of the individual course. Support groups can be very useful

because they provide the opportunity to meet those who have overcome eating disorders. These meetings help the client to broaden conceptions about him/herself, to inspire hope, and to feel unity with those who have already coped with the problem.

Conclusion

In PPT, eating disorders are categorized as psychosomatic. The three main forms of disorders—anorexia, bulimia, and BED—result from the processing in the body of problems arising in other spheres. Most often, the actual conflict is situated in the sphere of contacts. The emphasis on therapeutic relationships is central to the psychotherapeutic process. This is reflected in the importance of the development of the actual primary capabilities of the client and in expanding the system of relations that supports the client, both through the involvement of the family in therapy, and through the creation of an interprofessional team of specialists. The general vector of psychotherapy provides the transition from symptom to conflict through the understanding of the meaning and function of the disorder through positive interpretation, from the actual conflict to the basic conflict through the understanding of the dominant needs frustrating the life situation of the client, from the basic conflict to the internal conflict through the clarification and elimination of internal dissonances arising between the concepts of the client of him/herself, images of attractiveness, conditions of self-acceptance, and ways of coping with problematic situations. If these conditions are met, the approach of PPT becomes an effective way of helping the patient.

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Positive Psychotherapy in PTSD and Post-traumatic Growth

14

Tuğba Sarı and Ali Eryılmaz

Introduction

In the history of psychology, there have been many approaches to interventions for psychological disorders. One of these approaches is the positive intervention approach [1]. Positive Psychotherapy, founded by Nossrat Peseschkian (1977), has the characteristics of positive interventions mentioned in the literature [2]. The symptoms of clients are interpreted in a positive way, and their abilities—and capabilities—are studied in Positive Psychotherapy. This section first discusses trauma and post-traumatic growth. Then, the positive intervention side of Positive Psychotherapy (after Peseschkian) is emphasized.

Today, many individuals may experience one or more traumas in their lives. It has been found that 75% of people experience a traumatic life event during their life [3]. Traumatic events include a wide range of experiences, including terrorist attacks, wars, natural disasters, accidents, casualties, harassment, rape, bombing, wounding, and technological attacks. The word trauma, from ancient Greek, means "injury" or "to pierce." The word trauma was used to express the injuries that soldiers in ancient Greece receive from punctures in their armor [4]. For an event to qualify as traumatic, it has also been stated that it should have low controllability, be sudden and unexpected, create permanent or chronic problems, and have similar characteristics [5]. In the literature, traumas have been divided into two groups: micro (small) and macro (big) traumas [6, 7].

Traumas that cause a shock to the individual, such as sexual or physical attack, are considered macro traumas, whereas traumas such as humiliation, loss of life, and neglect are characterized as micro traumas. There may be significant differences between people's responses to traumatic events. In some cases, particularly after severe traumas, syndromes such as anxiety, depression, and post-traumatic stress disorder (PTSD) may emerge. Focusing only on the negative consequences of trauma can lead to a biased understanding of post-traumatic reactions [8]. Recent studies of traumatic events have shown that trauma has positive effects as well as negative effects [9]. For example, despite experiencing the same event in a traffic accident that results in an injury, each individual's reaction to the incident may vary. Some individuals continue their normal daily life in a matter of weeks and state that

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_14

it was an empowering experience, while some individuals may have various symptoms that affect their daily life and experience, such as mental illness.

In published articles, many clinicians and researchers have suggested that not only negative changes but also positive changes could occur from traumatic events [5]. Tedeschi et al. [10], for the first time, used the term post-traumatic growth to describe changes that have behavioral consequences due to highly challenging life events. Difficulties and suffering may find a relevant place in many belief systems and religions, such as branches of art, Christianity, Islam, Hinduism, and Buddhism. In these belief systems, the contribution of suffering to human life has been noted in many places. Parallel to this idea, existential philosophers such as Kierkegaard and Nietzsche emphasized that a traumatic experience is an experience in which people question the meaning of life [11]. Professionals in the field of practice stated that when working with trauma victims, their clients felt that they were now closer to people, they also started to help other people more, and they were trying to receive more help from people [12].

The construct of post-traumatic growth (PTG) suggests that survivors of traumatic events can not only heal from their trauma but may actually grow into stronger, more driven, and more resilient people because of their trauma. According to the latest research in positive psychology, enormous growth and development have been observed after unimaginable suffering, trauma, and pain [12]. It was observed that practitioners and researchers working in the field of positive psychology and positive psychotherapy focus on post-traumatic development. Nossrat Peseschkian [6] introduced the theory of micro and macro trauma to Positive Psychotherapy (PPT), which was developed in Germany in the 1970s. PPT provides a five-step intervention method in which the trauma is positively interpreted and rebalancing the life of the individual is aimed at empowerment and growth. Parallel to Nossrat Peseschkian, practitioners working in the field of positive psychology, which was developed in the late 1990s in the USA, have started to develop emerging models to prevent negative post-traumatic consequences [12, 13]. The aim of this section is to introduce interventions and examples based on Positive Psychotherapy after Peseschkian. The main focus of PPT is based on the principle of hope and finding the positive aspects of the client and the situation, reinterpreting the trauma, and, ultimately, helping bring balance to the client's life.

Definition of Key Terms

Post-traumatic Stress Disorder (PTSD)

PTSD is one of the most common psychological disorders following traumatic experiences. The symptoms of PTSD listed in the *Diagnostic and Statistical Manual of Mental Disorders* (DSM 5) published by the American Psychiatric Association are divided into four categories: (1) relieving the traumatic experience—intrusive symptoms; (2) avoiding reminders of trauma; (3) negative alterations in cognition and mood; and (4) hyperarousal and sensitivity [14]. To be diagnosed with PTSD, the individual must be experiencing the following symptoms for at least 1 month: (a) at least one intrusive symptom, (b) at least one avoidance symptom, (c) at least two cognition/thinking and mood symptoms, and (d) at least two arousal or reaction symptoms. Not everyone who experiences a traumatic event develops PTSD, but it is certainly not an uncommon diagnosis. According to the National Center for PTSD [15], approximately 7 or 8 people out of every 100 experience PTSD at some point in their lives, including approximately 10% of women and approximately 4% of men.

Post-traumatic Growth (PTG)

The positive changes following trauma have many names in the literature. These names include post-traumatic growth [11], personal growth [16], perceived benefits [17], positive reappraisal [18], and positive changes [19]. At its core, post-traumatic growth is basically a concept that points to an increase in individual functionality after a trau-

matic situation [20]. PTG has been operationalized by five major positive changes that manifest in the individual: (a) improved interpersonal relationships, (b) openness to new possibilities, (c) a greater appreciation of life, (d) an increased sense of personal strength, and (e) spiritual development [11]. According to the Posttraumatic Growth Research Group at the University of North Carolina at Charlotte [21], PTG can be conceptualized as positive change resulting from an individual's struggle with a major life crisis or traumatic event. This positive change typically manifests in one (or more) of five areas: (1) a sense of new opportunities or possibilities in life, (2) improved relationships with others, (3) increased mental and/or emotional strength, (4) greater appreciation for life in general, and (5) spiritual deepening (2014). Research has shown that PTG has numerous benefits, including lower levels of PTSD and higher levels of emotional maturity [22]. The theory has provided hope to countless people and spawned multiple resources and methods of facilitating healing and growth.

Balance Model of Positive Psychotherapy

According to PPT [23], there are four dimensions of life: body, achievement, contact, and meaning/spirituality. Individuals cope with conflicts in their bodies, achievement, social contacts, and meaning in life/spirituality. The purpose of PPT is to help the client reach a balance in these four areas and to widen the opportunities through which he or she copes with conflicts (see Chap. 2 for further discussion).

Five Stages of Positive Psychotherapy

PPT involves a five-stage process with clear psycho educational features [6]: (1) observation, (2) inventory, (3) situational encouragement, (4) verbalization, and (5) broadening of goals. This process gives structure to the therapy and can be learned by clients so that they can become their own "therapist" when the therapy ends (see Chap. 2 for further discussion).

Applications

Reestablishing Balance

While people are adapting to their lives, they face constant threats to their own balance. Piaget [24] states that people incorporate new knowledge and situations through assimilation into schemas. Thus, adaptation takes place and individuals reach equilibrium. The balance between assimilation and adaptation helps individuals to mature psychologically. In traumatic situations, individuals have difficulty incorporating the new situation they face. For this reason, traumas prevent individual development [18]. Coping with trauma also means that individuals might need to create a new level of balance.

In terms of approach, PPT focuses on the balance creation process. In the process of creating balance, there are many tools available in PPT. The most important of these tools is the balance model ([23]; see also Chaps. 2 and 8). The balance model proposes that people use resources to deal with conflict in a balanced way. Traumatic experiences disrupt the individuals' balance and narrow their resources for coping with conflict. In the psychotherapy process, it is an important objective to expand individuals' sources of coping with conflict [25, 26].

The post-traumatic growth process [5] is actually the process of rebalancing in terms of PPT. Individuals enter into a new balance of body, achievement, relationships, and spirituality. The dimension of *spiritual change* in post-traumatic growth corresponds to the fourth sphere of the balance model which is future/fantasy. A *character strengthening change* in post-traumatic growth corresponds to the body dimension as a source of conflict in PPT. *Relationships with people* in post-traumatic growth correspond to the relationship dimension in Positive Psychotherapy. The new possibilities in post-traumatic growth may correspond to other dimensions of the balance model and the success dimension.

Positive Interpretations of Symptoms

In the therapeutic process, one of the most important factors that foster this rebalancing process and increase post-traumatic growth is the positive interpretation of symptoms. Positive interpretation of symptoms means the expression of reality. In reality, there can be positive features or situations as well as negative features or situations [26, 27]. For example, *depression* is not considered to be merely a feeling of demoralization with a passive attitude. Depression also reflects the ability to react with profound sensuality. Similarly, *fear of loneliness* is not only inadequacy of self-management but also a strong expression of the need to engage with other people. In PPT, specific positive comments are suggested for some specific problems [6, 27]. In this way, determining the symptoms and problems individuals experience positively changes the perspectives of the clients in the PPT process. Clients feel themselves to be stronger at the end of this process. The level of acceptance of problems is increasing. Thus, this process uses the resources of coping with conflict more effectively [23, 28–30]. The following is an example of the process of creating balance by making positive interpretations to a client who witnessed the death of his father as he fell from a roof.

- *Therapist:* What makes you miserable?
- *Client:* Since I learned that my father fell from the roof, I can't go into the street. There's a revival/flashback.
- *Therapist:* Could you tell me a little about your experiences and feelings?
- *Client:* As I said, I left the house where my father fell from the roof. I never liked our house. It was too far away from the bazaar. It's been 2 months since we moved. If we had not moved, I would have constantly thought about it. My brother said I'm leaving this house, leaving the memories. Grandma said you don't like your dad. We've worked to overcome things. After my father's death, I left home and went to school. It was on the roof, it was in the house, and it was going bad on the street. We moved out. What my grandmother told us was that she left my father. However, we're away from my father's pain. It's weird how they stand. My grandma told us not to get over it.
- *Therapist:* To overcome this situation, what else can you do instead of being constantly upset?
- Client: In fact, I think to overcome the situation means to continue where it left off. Our new home is close to the bazaar and my brother's school. Even when my father was alive, we had to move out of that house. It's wrong when I think of my grandmother's attitude. Although we loved that house so much, we left the memories and continued our lives. This means we have overcome this situation. This house was my grandfather's house and was entrusted to us. No one can live in a house where his father died. To cope is to put aside all experiences and to move on from where you left off.
- *Therapist:* We talked about you constantly blaming yourself for what was happening and repressing your feelings. In fact, if you think of your situation as an opportunity for your development, what do you think?
- *Client:* I have to be careful about my father. You should not suppress emotions but use them correctly. Be a master of your emotions!
- *Therapist:* You better manage your emotions and make more sense.
- Client: Yes certainly. My father's fall and death were unexpected. Maybe I've been overly emotional. It's ridiculous not being able to overcome the situation in regard to these feelings. These emotions are feelings that suddenly occur. I've actually had a condition. Death has a reason. Death occurred as a result of an accident. Searching for the perpetrator of death, the roof, no one is guilty. We've lost my father. We can't stop what's going to happen. Nobody is guilty.
- *Therapist:* The biggest indicator of this is the intensity of emotion. I think even attending your father's funeral is a sign of coping for you.

- *Client:* Yes, it made me feel better.
- *Therapist:* I mentioned earlier about the balance model. What do you say is something that will empower you if you interpret this situation in terms of spirituality?
- *Client:* According to my belief, we cannot prevent what should be. When death comes, it can come for a small reason. Allah decides who will die, when, and how. The only thing that can be called upon is prayer when my father comes to mind. My father won't come back if I blow the house up with dynamite. The best thing I can do is make a fountain.
- *Therapist:* What else can you say?
- *Client:* We have already lost my father. Everyone knows that. It means nothing is permanent. My father is not the only person who died in this world. Nothing should be considered permanent. I know the value of life. I realized there was a death. Now, when I think of my father's loss, I will be more comfortable.
- *Therapist:* Very nice. So, what do you say will empower you if you interpret this situation in terms of the body dimension of the balance model?
- *Client:* I have all my memories in that house. I was born in that house, and I had a lot of memories with my father. This is nothing but an overcoming. We didn't go where we left off. We changed our house. Most of the things we have done, for example, to rent a new home, to move from that home with a disturbing event. We relax when we stay away from stimulants. Emotions are erased when acceptance is needed. I can think of them. To commemorate, the event ends here. When I mention my father, I can think of his jokes and not his fall from the roof. I can control my emotions. We're injured in this case. If we do not dress this wound, there will be inflammation. This wound can progress to death. If we think of good things instead of remembering my father's death, we will heal the wound. From the positive point of view, the location of our house is very close to the bazaar and close to our relatives. Good things continue to happen. This is the missing part of humanity. We can't look at the big picture.
- *Therapist:* I noticed that the negative thoughts you experience are from the intensity of emotion.
- *Client:* These are intense emotions. I get used to them. I don't put my feelings on the object, and it's a childish situation.
- *Therapist:* You said very important things. Instead of negative emotions and thoughts, you can think of positive emotions and thoughts. So, you see the big picture better.
- *Client:* Certainly, I will do this.

Five Stages of Positive Psychotherapy in PTSD

The contribution of psychotherapeutic processes to individuals cannot be denied. Practices based on many psychotherapy approaches contribute to individuals' post-traumatic growth [31–34]. PPT is one of these approaches. The practice of PPT in accordance with the consultation principle contributes to individual post-traumatic growth. One of the important meanings of the consultation principle is to carry out the psychotherapy process in five steps. These five stages of treatment are the observation/distancing phase, inventory phase, situational encouragement, verbalization stage, and expansion of goals [6, 25, 27–29, 35].

Trauma studies based on psychotherapies emphasize the importance of post-traumatic growth and underscore an important point. Post-traumatic growth has two important dimensions: adaptive and non-adaptive. In psychotherapy studies, too much focus on growth may mean that individuals escape from the consequences of an actual trauma [32, 34, 36, 37]. On this point, a strong side of PPT is emerging. PPT is a psychodynamically oriented psychotherapy. Although PPT contains positive elements, its main purpose is to eliminate pathologies [6, 25–29, 38]. At this point, the five stages of PPT provide opportunities to work on post-traumatic growth and on other trauma outcomes. In this section, the five stages of PPT and examples of working with trauma are included.

Stage One: Observation/Distancing:

The main purpose of this step is to understand the client's situation and problems. The psychotherapist listens objectively to the client about his or her trauma experience. The therapy focuses on the client's needs and problems. Once the client's problem has been identified, the client's symptoms are examined within the framework of the balance model. An example of a psychotherapist's examination of the symptoms of trauma is given below.

- *Therapist:* Can you tell me about your problem?
- *Client:* When I was 4.5 years old, there was a major earthquake. Although I cannot remember exactly what I experienced during this earthquake, what it left me still affects me. I am 22 years old now. I can say that I've been experiencing the effects of trauma since my elementary school years. Especially when I sleep at night, I can leap to fear even when someone comes near me. There is a residual effect from that time. I'm afraid of tall buildings. However, this fear usually hits me when I sleep at night. I also think that every night I can live through the same things again.
- *Therapist:* How does this affect your body?
- *Client:* When I'm afraid, my body twitches and I feel like I can't move. I cannot speak. I close my eyes. My eyes open too much. In some cases, I feel cold and breathe. After some time, my head hurts.
- *Therapist:* How does this affect your perspective of the future?
- *Client*: I start worrying about the future. I'm even worried about where I'm going to sit. I start to be restless, and I get bored a lot. I feel like there's going to be something going on for a few days, and I can't make plans for the future. I can't plan where I will live. I can be uneasy when I think my loved ones may have the same problem.
- *Therapist:* How does this problem affect your relationship with people?
- *Client:* Generally, this problem does not affect my relationships because it appears at night. At night, when the trauma comes to mind, I can't sleep until the morning and I'm sleepless for days. I cannot communicate in a healthy way with people during the day. I wonder if they are around me. I can't explain exactly, so sometimes I get bored. I also don't want to talk when I'm badly affected. It affects me negatively when I cannot communicate with people in a healthy way.
- *Therapist:* How does this affect your success?
- *Client:* It doesn't affect my performance much. If I've been very busy, I'm going to be absent-minded because it can influence my exams on that day. I'm very pensive when I'm doing homework and I can't be myself.

During this process, clients are given various assignments. One of these assignments is writing a letter to their symptoms. This assignment is intended to create a motivation for clients to respond to their symptoms and to ask them to better understand the function of their symptoms. The following is an example of a letter to a client's symptoms:

> I don't know how to describe you. I've learned to live with you, or you're still trying so hard, I don't know. I experience you when I see you with trauma triggers. When I met you, I was very small, but I remember you like the first time. Who was crying, who was shouting, all in a corner of my mind? The sound of dripping water that I still don't know where it comes from still annoys me more. When I hear the sound of water choking in the quiet place, in a silent place, I still remember you. You impress me a lot. When I meet with your triggers, my body twitches and I feel like I can't move. I don't often talk. Either my eyes open like a fortune cookie or they're tight. I'm cold, and my breath shrinks. After a while, my head hurts a lot. People around me wonder what I'm going through. I can't tell people about it, and I'm bored. I also don't want to talk when I'm badly affected. This affects me negatively because I cannot communicate with people in a healthy way. You don't affect my achievements much. If I live very intensely, I will be absent-minded because you can influence my exams on that day. I'm very pensive when I'm doing homework, and I can't be myself. I start worrying about the future. I'm even worried about where I'm going to live. I'm starting to become restless. I'm bored. I feel like something's going to happen. Sometimes it feels like I can't make plans

for the future. I can be uneasy when I think that my loved ones may have the same problem. Natural disasters can happen anywhere in the world. I can live like this all the time. Everyone can live. I need to remain calm and natural. I need to pray to Allah to be protected from you. God will make the most auspicious thing about me. I pray.

Stage Two: Making an Inventory

In this process, the balance model is used to inform the client about what he or she has been experiencing in the last 5 years. It is used to try to understand how the client has solved the problems he or she faces related to the trauma and which dimension of the balance model he or she uses more. The levels of primary and secondary capacities are investigated. In this process, the client's micro-traumas are also examined. At this stage, the therapist begins to formulate hypotheses about the actual conflict (the traumatic event that happened recently) and the basic conflict (the traumatic event that happened in early childhood).

Figure 14.1 illustrates the state of the client in terms of capacity. The client states the situation: "When I think of my traumas or when I meet with their triggers, I feel helpless and fearful. In this process, my ability to hope comes into play and reminds me that I am not helpless and that I am safe. My faith also supports my ability to hope. In this process, the ability to love creates fear in me

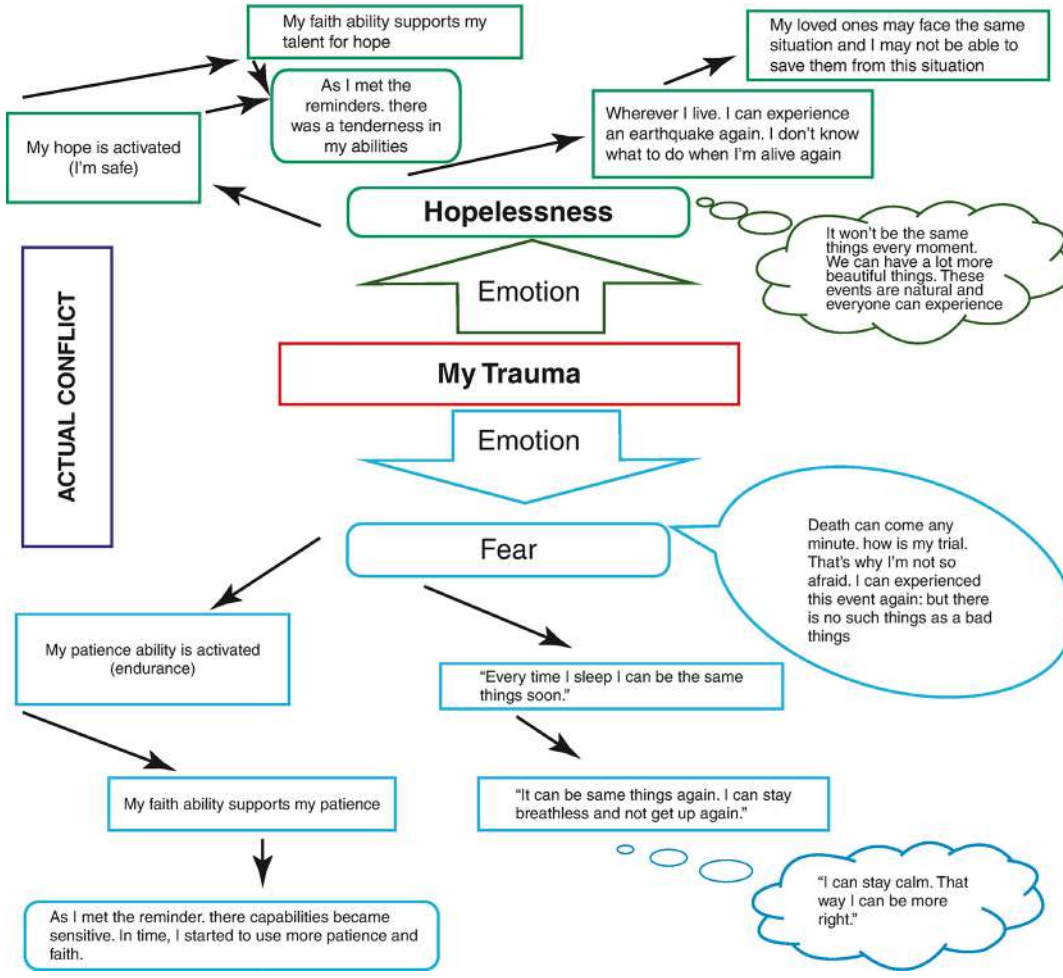


Fig. 14.1 An example from a client's actual abilities to cope with trauma

because it makes me think that I can lose my loved ones due to a possible earthquake. As I meet the triggers, I am trying to cope with them with patience and faith."

Stage Three: Situational Encouragement

Situational encouragement is of central importance in PPT. Since the client focuses only on his or her problems, he or she cannot realize the positive aspects and opportunities. In this context, the client is encouraged on the positive aspects of the situation from a more realistic perspective. An attempt is made to help the client find new alternative ideas to expand his horizons. Below are positive interpretations from group members in a PPT group for a member who experienced a traumatic experience.

- *Therapist:* What are the positive features you see in our friend?
- *Feedback from the group members:* She is patient, calm, friendly, beautiful, energizing, and also knows how to listen. She is also funny, remains calm, helpful, clean, mature, acts according to place, and a compassionate mother.
- *Therapist:* What kind of emotions and thoughts did your friends' feelings give you?
- *Client:* My friends' list of positive qualities made me feel good.
- *Therapist:* Very nice. Now give feedback about what our friend's problem contributed to her?
- *Group members' feedback:* It has changed her perspective on life. She increased her religious belief. She understood the importance of being safe. She understood the value of their relationship. It didn't affect her relationships.
- *Therapist:* What kind of emotions and thoughts did your friends' feelings give you?
- *Client:* I am strong. When my friends spoke, I realized my positive qualities once again. I'm a woman who can stay calm and think maturely. I can stay calm when I think about my trauma. I can remember how to cope with mature thinking. I'm a patient. I can deal with my trauma using this trait. I think it's the best way to handle patience. I have this property. I have the power to deal with my trauma. God has always been with me and has given me many positive features. I'm dealing with my trauma by using them.

Stage Four: Verbalization

In previous stages, after creation of the necessary therapeutic environment, the basic conflict (the trauma happened in early childhood) of the client is studied in this period. An attempt is made to help the client solve the actual and basic conflict by using the balance model and its four dimensions. The main purpose of this circuit is to focus on the ways in which the client can balance the four dimensions of his life. This period is the longest in the consultation process and continues until the basic conflict is resolved. The following is an example of an action plan used in the verbalization step when intervening after a trauma. An example from a client's action plan is shown in Table 14.1.

Table 14.1 Example of an action plan used by the client to deal with trauma

	My activities according to the action plan									
Days of weeks	Scenario (I'm worried/I'm comfortable)	Behavioral control technique intervention	Balance model	Letter of separation suggestion	Dealing with faith	Editing cognitive distortions	Upload meaning to triggers	Edit my basic conflicts	I don't carry my trauma with me (story of the traveler)	Situational encouragement (positive suggestions)
Monday	X					x				
Tuesday		x	x							
Wednesday				x						x
Thursday							x			
Friday					x			x		
Saturday									x	
Sunday					x					

Stage Five: Broadening of Goals

This stage focuses on how the client wants to live in the future (in the short to medium-long term) after reaching the objectives. The main purpose of the psychotherapeutic relationship in this circuit is to provide information to the client about self-help. In this way, the person can be equipped with a self-help method to accompany him/her during his/her life after ending therapy. In this stage, the client learns to reconsider the goals he or she pursues before he or she begins his or her life again. In doing so, he or she takes the four dimensions of life in the balance model and plans how to maintain balance in his or her life. This stage usually includes short-term therapy of one to two sessions and then long-term therapy that lasts four to five sessions.

Usage of Stories

Another original intervention of PPT is the usage of stories in the therapy process (see Chap. 36). Stories in proverbs, wise sayings, and metaphors are used as mediators between the client and the therapist. Story use has many functions in the therapeutic process. These functions include being a mirror and identifying and transferring the past to the future [39], p. 7; [40]. The aim of the stories is for the client to look at the trauma or the problem through a new frame and a new spectacle with the help of the story. In the example given below, the therapist presented the story of a traveler [39] to the client and asked him to write down his thoughts about the story.

> ***I do not carry my trauma with me.*** There is no requirement that I will always experience positive things. Sometimes I have to experience negativities. I may have had a trauma. I may have had a hard time coping with my trauma. That's perfectly normal. I've been carrying it all the time with me because I didn't know how to handle it. I'm traumatized. However, I know how heavy it was. I couldn't afford it. I was always worried, and I was on top of it. At home, at school, the fear that I would live the same everywhere I went was wearing me out and I was getting tired. On the other hand, I knew that it was a test. Why am I carrying it then? Why was I persecuting myself? It was an unnecessary burden for me. I shouldn't have moved it anymore. That was much better. I don't move you anymore. You haven't left me for years, but I grew up with you and I don't want you anymore. I don't want to be scared or anxious anymore. However, now that you've lived, you stayed in August. I leave you there. And I'm on my way. I'm happier like that without you. When I don't think about you, I'm enjoying more of my experiences. You're not with me anymore, even the main challenge. Allah knows best, and he will give me strength. I'm on my way now without you. Not happy with you. I've carried you as a burden so far. However, you're heavy now. I'm leaving you on the roadside in the heat of August. Hope to not meet again.

Summary and Key Points

Positive Psychotherapy (PPT after Peseschkian) is a therapy model that has proven to be effective for many psychological disorders [6, 25, 27–30, 41–43]. This therapy model can be used effectively both for individual and group psychotherapy for post-traumatic stress disorder. In a study of positive group psychotherapy with a heterogeneous structure, individuals who had trauma problems were found to be able to solve these problems [41]. Trauma is a psychological problem that can be experienced by individuals of all age groups and by those who are psychologically strong and weak [44]. At this point, while trauma symptoms are common, individual differences are observed in traumatized individuals. These differences are also reflected in the psychotherapy process. In this part of the chapter, suggestions are given for the five stages of therapy for practitioners who will work with trauma.

Observation/Distancing Stage

When working with individuals with trauma problems at this stage, it is necessary to explore the problems and needs of the clients. Subjective questions such as what, how, where, when, and who are used to understand the trauma and the individual in detail. At the same time, the trauma experiences of individuals with behavioral control techniques are examined as pre-event, during

the event, and post-event feelings, thoughts, and behaviors. The client's symptoms are recorded. A letter can be given as an assignment to the client.

Making an Inventory Stage

The therapist applies the Differentiation Analysis Inventory. The client's trauma is examined in terms of sources of coping with conflict. At the same time, the client's trauma is evaluated in terms of model dimensions. Micro-traumas they have experienced in the last 5 years are discussed. The client is asked to find a history related to his problem, or a story is recommended by the therapist.

Situational Encouragement Stage

Trauma is made of red flag symptoms that are given a positive interpretation. The positive characteristics of the client are emphasized. In particular, the client's situation can be evaluated in terms of post-traumatic growth dimensions at this stage.

Verbalization Stage

With the client, action plans are prepared for intervening in the trauma problem. These action plans can be techniques from other therapy models. The therapist has an educational role in the process of creating and implementing the action plan.

Broadening of Goals Stage

The expansion phase focuses on the client's aims, in which a life without trauma symptoms is made. Clients determine various goals in the areas of body, success, relationships, and spirituality after solving the trauma problem. The therapist works with clients on how to convey these life goals.

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Positive Psychosomatics

15

Ivan Kirillov

Introduction

Despite recent advancements in the psychosomatic understanding [1], three key challenges [2] persist:

1. *Patients' Excessive Fixation on Their Physical Suffering* and urge to control that can lead to resistance towards psychotherapy, hindering timely and accurate diagnosis and treatment.
2. *Lack of Systematic Description of Somatization Process.*
3. *Absence of Systematic Diagnostic and Treatment Strategy.*

Positive Psychotherapy effectively addresses these challenges by adopting a constructive and practical approach rooted in the *positive*[1] [3] and *salutogenic* [4] models. It views any psychosomatic symptom as a capacity to discharge the tension of inner and/or outer conflicts and to restore health. Nossrat Peseschkian [3, 5] introduced innovative tools to operationalize the psychodynamics of conflicts and somatization processes. His five-stage psychotherapeutic strategy offers a systematic framework to effectively engage psychosomatic patients, labilizing their fixation on physical symptoms and enhancing their capacity for mentalization.

Nossrat Peseschkian defines the field of *psychosomatic* as a "specific line of research and treatment that strives to illuminate the connection between the mental events and bodily reactions" [5]. He qualifies *psychosomatic disorders* as "bodily ailments and functional disorders of the organism, the etiology of which and course of which are largely dependent on social psychological circumstances" [5].

The Evaluation Criteria for Psychosomatic Practice (ECPP) [6] evolved from the Diagnostic Criteria for Psychosomatic Research (DCPR) [1] to integrate and operationalize the main known factors of somatization (Fig. 15.1):

[1]Positive /ˈpɒzɪtɪv/: consisting of or characterized by the presence rather than the absence of distinguishing features. (https://en.oxforddictionaries.com/definition/positive). /positum/ (lat.): placed, positioned (ABBYY Lingvo, Oxford Latin Dictionary, 1982; Latin Dictionary by Enrico Olivetti)

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_15

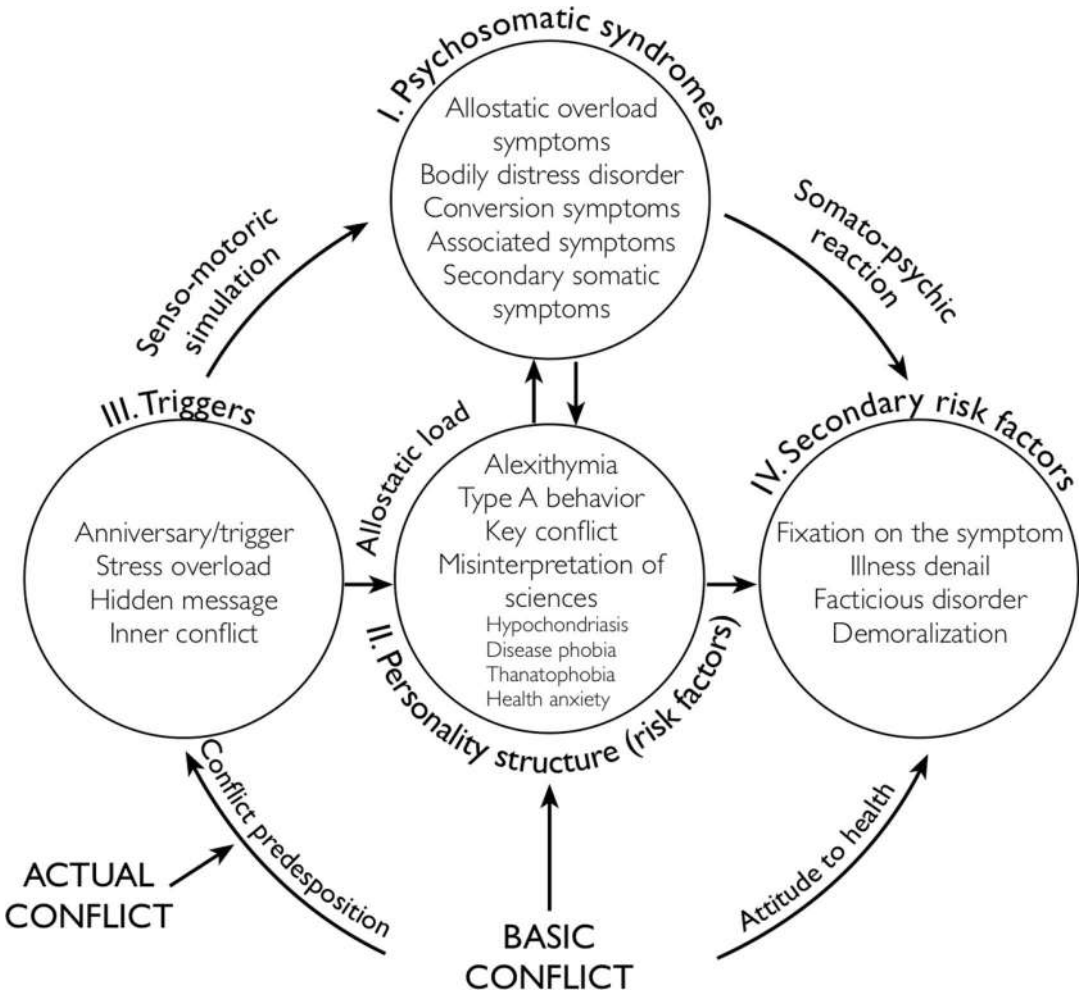


Fig. 15.1 Evaluation Criteria for Psychosomatic Practice (ECPP) in the dynamic of somatization

Somatization Factors

1. *Psychosomatic Syndromes*, developing according to recent theories, as a result of:
   (a) *Allostatic load* [7–10] occurs when "the cumulative burden of chronic stress" [11] generated by inner and outer challenges overwhelms an individual's resilience. This overload initially manifests as reduced impulse control (*Conversions*), then progresses to fatigue (*Allostatic Overload*), and eventually to persistent functional symptoms of *Bodily Distress Disorder*[2], and then to *Organic Dysfunctions*[3] (indexed in specific

[2] *Bodily Distress Disorder* [52] (ICD-11: 6C20; [53] ICD-10: F-45:3 Somatoform autonomic dysfunction; DSM-5: 300.82 Somatic Symptom Disorder). *In terms of Positive Psychotherapy, one can see this group of disorders as the capacity to indicate the inner conflict and tension of the autonomic nervous system.*

[3] *Organic Dysfunctions* appear when the chronic stress of the autonomic nervous system exhausts the physiological resilience and causes noticeable changes in the structure and function of the cells, tissues, and neuro-immune system [12], such as gastric and/or duodenal ulcer, Basedow's disease, Hashimoto syndromes, type 1 diabetes, functional heart disorders, headaches, intestinal disorders,

somatic rubrics). Additionally, early-life trauma and chronic stress can cause epigenetic changes, altering gene expression and potentially contributing to long-term health problems [15].

(b) *Senso-motoric simulations* in the cortex vividly reactivate[4] past bodily experiences [20–25] in the form of *Conversions*[5], *Associated Symptoms*[6], and *Secondary Somatic Symptoms*[7].

2. *Risks associated with personality structure vulnerabilities* [27, 28], hindering constructive processing of inner and outer stressors:
   (a) *Alexithymia*—"inability to express, describe, or distinguish among one's emotions" [29].
   (b) *Type A behavior:* workaholism, chronic competitiveness, high achievement and recognition motivation, impatience and a distorted sense of urgency, irritability, tension, accelerated physical actions (sudden movements, rapid speech) and mental processes, polyphasic activity (e.g., shaving or eating while reading a newspaper), aggressiveness, and cynicism.
   (c) *Key Conflict* (see Chap. 35 on the conflict model details) between *directness (openness)* and *discretion (courtesy)* [3, 30] regulates the processing of the tension [31] of the inner conflict, playing a crucial role in somatization, particularly when prioritizing discretion, restraining one's own reactions to secure a relationship, even at the expense of personal interests and comfort.
   (d) *Misinterpretation of Bodily Sciences:* hypochondriasis[8], disease phobia, thanatophobia, health anxiety.
3. *Triggers of stress*
   (a) *Stress overload* of psychosomatic resilience by accumulation of macro- and micro-traumas.
   (b) Emotionally charged *trigger, or anniversary* of death/terminal illness of a loved one eliciting a symptomatic reaction involving autonomic arousal, functional syndromes, or associated symptoms.
   (c) The *hidden message*, consciously or unconsciously conveyed by the symptom, which results from the constant suppression of emotionally charged desires (impulses), allows the individual to elicit desired responses/actions from others.
   (d) The *inner conflict of motivations* manifests itself in excessive emotional, behavioral, and mental reactions that occur in different contexts but are always triggered by a specific trigger (*"every time I am required to obey, I explode with anger"*)

rheumatic problems, asthma, etc. The specificity of the organs to suffer is defined by stress, genetic predisposition, and specific life conditions rather than the psychological content. Yet the retrospective interpretation of the symptom's symbolic [13] or functional meaning allows the patient to reintegrate somatic experience by working through the conflicts to discharge them [14]. *In terms of Positive Psychotherapy, it can be seen as the capacity to indicate the suppressed or unattended inner conflict and exhaustion.*

[4]The preliminary results of neuroimaging studies show that during conversion, the primary perception remains intact, yet the activation of the insula [16], orbitofrontal and anterior cingulate cortex [17, 18] and limbic system act via inhibitory basal ganglia–thalamocortical circuits to reduce conscious sensory or motor processing [19].

[5]*Dissociative* ([52] ICD-11: 6B60.Z)*/Conversion neurological symptom disorder* ([53] ICD-10: F-44) or *Functional Neurological Symptom Disorder* (DSM-5: 300.11) is the physical form of regression due to intense stress, anxiety, or emotion [26], arising from unconscious conflicts and, therefore, easily acquiring a symbolic significance [13]. *In terms of Positive Psychotherapy, one can see this disorder as the capacity to express and discharge the tension of the inner conflict through the body.*

[6]*Associated symptoms* occur in response to a specific anniversary or trigger and resemble the symptoms of physical illness experienced by the patient and/or observed in someone else or wished on someone else.

[7]*Somatic Symptoms Secondary to Psychiatric Disorder* develop after the onset of a psychiatric disorder and are related to that disorder (e.g., heart palpitations due to pre-existing panic disorder).

[8]*Hypochondriasis* ([52] ICD-11: 6B23: [53] ICD-10: F-45.2: Hypochondriacal disorders; DSM-5: 300.7 Illness anxiety disorder). *In terms of Positive Psychotherapy, it can be seen as the capacity to look for the more profound problem in health instead of the psyche.*

related to the protagonist's unconscious needs and their collisions.

4. *Secondary risk factors*[9]—somatopsychic reactions encompassing an individual's psycho-social response to bodily symptoms, influenced by their personality, beliefs, and inner conflicts. These coping mechanisms can either aid or hinder treatment effectiveness, potentially leading to chronic conditions and/or comorbid mental health issues such as *Demoralization* [32, 33], *Symptom Fixation* [34], *Illness Denial* [11, 35], and *Factitious Disorder*[10].

Somatization Process

Psychosomatic Arch was first introduced by Nossrat Peseschkian in 1988 [38] to help both the patient and the therapist visualize and thus understand the process of somatization in order to find the best interventions to cure psychosomatic disorders.

The Psychosomatic Arch was further developed [38–42] in line with new findings of neuroimmunology and neurophysiology within the recent decades (Fig. 15.2).

The process starts with the *Actual Event*.

1. The central nervous system (CNS) rapidly perceives external or internal changes through sensory receptors, triggering a stress response within half a second.
2. The brainstem's reticular formation (*vegetative level*) "turns on" the sympathetic nervous system by releasing adrenaline into the synapses. General arousal, aggravation of attention, vasoconstriction, and increasing heart rate and respiration prepare the body for action.
3. Almost simultaneously, the *limbic system* assesses the perceived danger by comparing sensory data with *"emotional memories"*, activating corresponding emotions to channel the energy of stress into the impulse that can be rapidly discharged through involuntarily motoric reactions.
4. The evolutionarily newer *cortical structures,* particularly the prefrontal lobe, operate at a slightly slower pace [43, 44]. Initially, even before awareness, the ventromedial prefrontal cortex (vmPFC) predicts biographically associated emotional outcomes of perceived events and one's own impulses, generating the "gut feeling" of "*how will I feel if…"* and encouraging to enact "good-felt" impulses or suppressing the "bad" once by means of the dorsolateral prefrontal cortex (dlPFC) [45].
5. Further, cortical structures recognize the formed readiness to act or the suppressed impulses and process them in one of the following ways:
   (a) *Enact an intention,* discharging the impulse if the forecasted outcomes subjectively seem not dangerous and/or promising.
   (b) *Activates the "search engine" of thinking (What? Why? How? Why? When*?) when information is insufficient or new and unknown.
   (c) *Triggers the Inner Conflict*[11] if the situation habitually evokes the *preexisting*

[9] *Psychological or behavioral factors affecting disorders or diseases classified elsewhere* ([52] ICD-11: 6E40; [53] ICD-10: F-54; DSM-5: 316) are the psychological and behavioral reactions that affect the course and treatment of physical illness, such as avoiding care, demanding extra care, etc. *In terms of Positive Psychotherapy, one can see this disorder as the capacity to deal with the anxiety related to illness.*

[10] *Factitious Disorder* (F68.10 [300.19]) ([52] ICD-11: 6D5: [53] ICD-10: F-68.10; DSM-5: 300.19) imposed on self on another. *In terms of Positive Psychotherapy, it can be seen as the capacity to demand care or to attract attention, even without apparent benefits.* Interestingly, it triggers the same changes in neuroimaging as conversion and hypnotically induced paralysis [36, 37].

[11] *Inner conflict (IC)* usually happens unconsciously here and now as a clash of an actualised need with an event (*Actual Conflict)* perceived (because of the conflict potential formed in the Basic Conflict) as a threat or opportunity to meet this need.

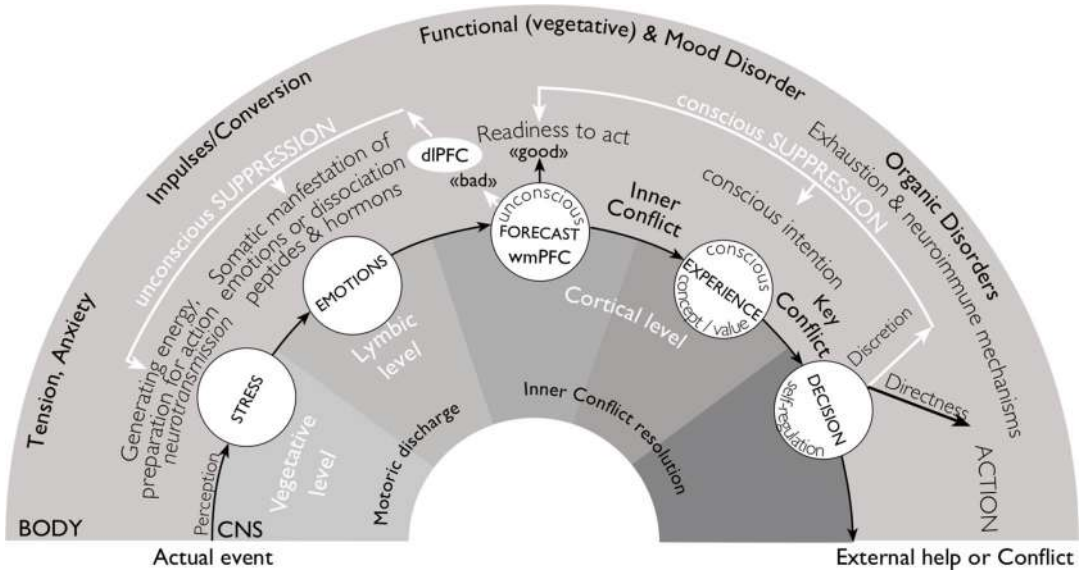


Fig. 15.2 The Psychosomatic Arch. (Modified by I. Kirillov after N. Peseschkian, A. Remmers [38, 42], Boncheva [39])

concept(s) modeled in the *Basic Conflict*[12]. Then one subjectively perceives the happening as a threat to one's primary needs or as a chance to meet them (*"I'm afraid to make a mistake because then nobody will love me", "this is so unpleasant, but as a good person (ideal) I have to do this"*). Such perception amplifies the emotional impulse to protect the activated primary needs, conditions stress to become chronic and therefore causes functional bodily and mood disorders.

6. The emotional impulse generated by the *Inner Conflict* or "*search engine"* triggers the *Key Conflict ("to do or not to do")* and, depending on its habitual processing, learned in *Basic Conflict* initiates one of the three possible scenarios:
   (a) *Constructive* use of the primary capacities to notice, understand, and reorganize the *Inner Conflict* [46] discharges the tension, and all systems restore normal functioning.
   (b) *Straightforward* outburst of unprocessed tension of the *Inner Conflict* outside into the attempt to meet one's own inner needs employing the external objects.
   (c) *Courteous* suppression or *Ambivalent* nonattendance [47, 48] of unresolved *Inner Conflict* continually builds up the chronic stress that, in the case of genetic predisposition, can cause the neuroimmune system to reduce immune activity, opening the possibility for infections or, contrary to trigger the excessive or one-sided immune reaction that might start to attack the tissues of different organs such as thyroid (Basedow's dis-

[12] *Basic Conflict (BC)* of the child's needs against the needs and abilities of primary caregivers begins in early childhood.

Later, it develops into emotionally meaningful relationships based on transference. Dealing with the BC, one internalises a predisposition for the conflict (bodily reaction, the conceptualised value of certain norms and behaviours, ways of coping, expected change in relationships).

ease and Hashimoto syndrome), pancreas (Diabetes type 1), connective tissues (Bechterew's disease), and others.

Therapeutic Applications

The five-stage strategy of positive psychotherapy for psychosomatic disorders provides a systematic and sequential framework to address the critical processes involved in the above-described pathogenetic developments.

Stage One: Observation/Distancing

At this stage, the therapist fosters the trust and engages the patient in the therapeutic process by:

1. *Observing* the patient and their body language.
2. *Actively listening* to the patient's concerns, complaints, and life experiences, both verbally and nonverbally, paying attention to what is said and what is omitted.
3. *Mindfully managing their (therapist's) own emotions*, ensuring they don't overwhelm the client's experience, while strategically containing them to enrich the therapeutic process [49].
4. *Utilizing the balance model* (Fig. 15.3) [3] *to structure the therapeutic dialogue* and guide the systematic exploration of the patient's experiences (*What exactly do you feel? How does it affect your general physical state? … your productivity? …relationships? Fears and expectations?*). Empower patients to internalize this model for ongoing self-reflection, memory reframing, and transforming their self-perception.
5. *Facilitating patient engagement* by creating a non-judgmental bios-free space for open communication, encouraging the expression of feelings, thoughts, and doubts, and using the patient's own language, symbols, and metaphors for questions, reflections, and interventions.
6. *Repeating, summarizing, and paraphrasing the patient's words* to ensure accurate understanding and encourage deeper exploration of their experiences.
7. *Shifting the patient's focus from somatic symptoms* by systematic examination of them in relation to triggers/stressors, specifics of personality structure, and secondary reactions to the bodily symptoms (Fig. 15.1). When appropriate, encourage the patient to envision desired health outcomes, focusing on expected experiences and sensations of health rather than simply negating illness symptoms (*e.g., instead of "the pain will go away," ask them to describe how it would feel without pain*).
8. *Focusing on capabilities* (norms and behaviors) that trigger stress and inner conflicts and thereby weaken physical resources and contribute to psychosomatic reactions. This encourages the client to shift attention from the somatic symptom to the behavioral and psychological aspects of daily stressors.

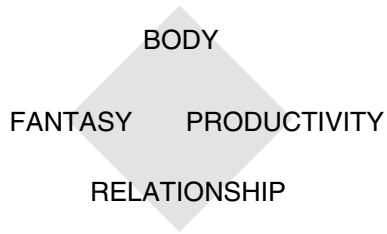


Fig. 15.3 The balance

Psychodynamic Focus The positive psychotherapist engages in ongoing mental experiments, trying to understand the symptom as a patient's subconscious attempt to discharge the suppressed impulses of inner conflict and stabilize or develop existing social systems. Such a positive focus of attention boosts the ability of the therapist to offer the patient secure eye contact and emphatically mirror their emotions and states while keeping an observing distance from the patient's neurotic experiences and from one's own countertransference. The therapist validates the patient's narrative and unconscious processes with a containing compassionate smile, always striving to discover the positive

function of the symptoms and their symbolic meanings.

The therapist's positive attitude provides the patient with an alternative psychodynamic experience of the symptoms: curiosity vs avoidance, understanding vs shame, developing capacities vs anxiety, and hope vs despair. Such a change of emotional experience affects the immediate reactions of the brain stem and, therefore, can reorganize the emotional memories if it continues long enough (approximately six weeks).

If appropriate, the therapist employs humor, alternative viewpoints borrowed from folklore and different cultures, and general positive interpretations suggested by N. Peseschkian [5] to help the patient gain a new perspective on his/her disorder (*e.g., heart attacks—the ability to take burdens and risk factors to heart; impotence—the ability to withdraw from the arena of sexual conflict*).

Self-Help Toolkit

- Self-Observation: *list your daily stressors, describe your actual reaction, and suggest a desirable alternative reaction to each of them.*
- *List your capabilities and strong sides.*
- *List important life events that you experienced and measure them against the Social Readjustment Rating Scale (SRRS)*[52].
- The Balance of Life: *how do you distribute 100% of your life energy among the four areas of the balance model?*
- The balance of Reactions: *reflect your reactions towards your symptoms in 4 areas of life.*
- *What can your symptoms symbolize? What can be a symbol of your symptom?*
  What can be a symbol of your health?

Social Adjustment Interventions

- *How do the other people in your life react to your somatic conditions?*
- *How do the other people in your life cope with similar situations in their lives?*



Fig. 15.4 Four dimensions of relationships/modeling on this picture PRIMARY WE' should be clearly in the left corner of the diamond, and 'YOU' should be clearly in the right corner, and there should be a clear space between them (as in uploaded file)

Stage Two: Inventory

This stage aims to boost the patient's awareness of the function of their somatic symptoms in the psychodynamic context of their biography and motivational conflicts, causing chronic stress.

Therefore, the therapist uses questions to engage the patient in exploring the role of triggering norm(s) (secondary capability(s)), key conflict (directness and discretion), and health in relationships with the primary caretakers in the basic situation of childhood and (if applicable) in their present relationships. This inventory is to be guided by the four dimensions of relationship/modeling (Fig. 15.4).

I-relationship

- *How did your parents/siblings treat you in childhood?*
- *What role did [triggering norm, directness/discretion, health] play in the attitude of your parents/siblings toward you in childhood?*
- *What role does [triggering norm, directness/discretion, health] play in your (other's) attitude toward yourself?*

YOU-relationship

- *How did your parents treat each other?*
- *What role did [triggering norm, directness/discretion, health] play in the relationship of your parents with each other?*

– *What role does [triggering norm, directness/discretion, health] play in your relationships with your partner?*

WE-relationship

– *What were the relationships of your parents with their social environment?*
– *What role did [triggering norm, directness/discretion, health] play in the relationships of your parents with their social environment?*
– *What role does [triggering norm, directness/discretion, health] play in your relationships with your social environment?*

PRIMARY-WE-relationship

– *What did your parents believe?*
– *What role did [triggering norm, directness/discretion, health] play in the belief system of your parents?*
– *What role does [triggering norm, directness/discretion, health] play in your belief system?*

The therapist helps the patient discover his/her personal dynamic of forming individual resources and coping strategies:

– The *subjective concepts attributing the emotional value* to the secondary capabilities (identified as triggers on the observation stage), especially *directness, discretion, and bodily symptoms*, depending on how they helped or hindered the satisfaction of vital and primary needs in a *basic conflict*.
– The *primary bodily reactions (stress, emotions, symptoms)* aimed at surviving and satisfaction of vital and primary needs.
– The sufficiency level of *primary capacities* of personality structure to regulate the inner mental processes and conflicts in order to meet corresponding primary needs.

Such an interview, supported by the Differentiation-Analytical Inventory (DAI) [5], helps the patient realize that his/her conflicting attitudes, which seemed to be immutable and personality-bound, are indeed biographically conditioned attempts to solve some inner or social conflicts that can be more painful than the physical suffering.

Psychodynamic Focus The therapist uses his/her primary capacities and structured interview to help the patient differentiate the contents and dynamics of their motivations and inner and outer conflicts leading to chronic stress and, thus, to psychosomatic. Besides, this discussion develops the patient's capacity for mentalization.

Self-Help Toolkit
– Differentiation-Analytical Inventory (DAI) [5].
– Wiesbaden Inventory for Positive and Family Therapy (WIPPF) [53].
– Stress Surfing Journal *(identify triggered primary needs by behavioral reactions)* [42].

Social Adjustment Interventions
– *What does the [symptom] mean for you/for your partner, friends, people from different cultures?*
– *What functions does the [symptom] perform in your life/in the life of your partner?*

Stage Three: Stimulation (Situational Encouragement)

At this stage, the therapist uses the:
1. *Paradoxical encouragement of the symptom* by the verification (following the client's capacity of mentalization) of the understanding of its positive function:
   (a) for clients with developed mentalization: *"Am I getting it right that you perceive any requirement of obedience [situation described in terms of secondary capabilities] as the threat for your contact with yourself [primary need] and react with the intense anger and*

impulse to rebel [level of stress, emotion, behavioral impulse] to protect/meet your contact with yourself [primary need]?
If the client confirms this, then continue with the Key Conflict if appropriate:
"It seems that you perceive your impulse to rebel [description of the impulse] as the threat to your need to be a good (ideal) son [primary need] and suppress your impulse (discretion) to protect/meet your need to be good (ideal) son [primary need].
If the client confirms this, then continue with the interpretation of the symptom's function if appropriate:
"And then your body continues to generate the energy to protect your needs and discharge it through the asthmatic attack [somatic symptoms]?"

(b) for clients with poor mentalization:
"Am I getting it right that your asthmatic attack [symptom] is your capacity to:
protect your contact with self and still be a good son [primary need]?
tell parents that you cannot do what they ask [implicit message]?
ask for freedom to stay in contact with yourself [needed help]?

This intervention is meant to condition the client's insight into the inner conflict and function of the symptom, discharge defenses and condition the active search for alternative reactions.

2. *Situational encouragement*—regular positive attention to abilities revealed in life situations and during the session. This focuses the client on what is already achieved and available as an active resource and generates self-efficacy, confidence and impulse to act.
3. *Psycho-serum (antidote)* can be used to facilitate the "inscription" to the automatic subconscious memory of newly gained positive perception of the symptom and positive counter-concepts. Technically, this can be done by hypnotic suggestion, autogenic training, guided imagery, stories, symbols, metaphors, humor, etc.

Psychodynamic Focus The therapist provides the patient with the model of finding and trusting resources, encourages self-help, and conditions the inspiration and curiosity to experiment.

Self-Help Toolkit
- *What positive influence do your symptoms have on your environment?*
- Breathing and relaxation exercises
- Autogenic training

Social Adjustment Interventions
- *What positive influence do your symptoms have on your environment?*
- *How can you establish trust with the conflicting partner?*
- *What of his/her capabilities can you encourage? How?*
- *How can you show him/her the understanding of those behaviors that disturb you?*
- *Who can help/encourage you? How can you ask for this?*

Stage Four: Verbalization

Focus the discussion on explored and overlooked conflicts to *ensure the patient's clear understanding of the inner conflict,* its origins, and its role in the development of psychosomatic symptoms. Utilizing the ECPP and/or Psychosomatic Arch, presented in a simplified and tailored manner, can aid both therapist and patient in visualizing and comprehending the dynamics of somatization, while also offering insights into potential paths towards restoring psychosomatic health.

Address the key conflict by noticing its manifestations in the therapeutic relationship and discussing its origins and role in the development of chronic stress and psychosomatic symptoms.

Condition the constructive resolution of the key conflict by:

- Listening and questioning while the patient is emotional.
- Being direct, clear and sensitive to the patient's lexicon and level of understanding in your questions and comments.
- Focusing the questions and comments not on the person, but instead on actions and capabilities to stimulate the natural impulse to develop his/her capabilities and to test new behaviors.

Ask the patient to come up with *alternative practices* that can:

- Optimize his/her concepts and perceptions.
- Help him/her to explore the ways to meet the triggered needs constructively.
- Develop vulnerable primary capacities of the patient to boost her/his self-sufficiency.
- Encourage the patient's responsibility for future physical, emotional, social, and behavioral development.

Psychodynamic Focus The therapeutic relationships serve as an active model of the balance between *directness* and conscious responsibility for one's own emotional reactions (*discretion*).

Self-Help Toolkit

- *How often does the conflict of directness vs. discretion (courtesy) arise, and with whom?*
- *What needs of yourself, and your partner does it trigger?*
- *How can you now explain your problems/needs to your partner?*

Social Adjustment Interventions

- *How can you verify your understanding of the problems/needs of your partner?*
- *What mutually beneficial solutions can you offer to your partner?*
- *How can you agree with your partner that inconsideration hurts you and him/her more than a frank conversation at the right time?*

Stage Five: Broadening of Goals

At this stage, the therapist focuses on the:

1. Feedback from the patient to activate the impulse to act.
   - *What have you already achieved with the therapy?*
   - *What was important for you?*
   - *What will you do now?*
   - *What do you want to bring to your colleagues, family, and friends?*
   - *What would you like to discuss at the next session?*
2. Developing the patient's underdeveloped capacities, especially the primary ones.
3. Building the life balancing plan (one to three objectives supported with three to five specific activities) using the model of balance and four dimensions of relationship responsibility (I, You, We, Primary-We) for 5 years, 1 year, 1 month, 1 week, 1 day.
4. Exploring and experimenting with the symptoms of health.

Psychodynamic Focus To strengthen the new ideas by "installing" them into the client's inner "story" to balance the vision of a healthy future and make it more realistic.

Self-Help Toolkit

- *What would you do if you had no more symptoms?*

Social Adjustment Interventions

- *What would you do with your partner if you had no more symptoms?*

Summary

Positive Psychotherapy effectively addresses all three challenges of contemporary psychosomatic medicine.

1. A positive understanding of psychosomatic symptoms as a capacity to physically discharge the inner tension of the underlying inner conflicts, accompanied by the balanced questioning helps to engage the patient into the therapeutic collaboration, gain his/her trust, and labialize the excessive somatic fixation.
2. The Evaluation Criteria for Psychosomatic Practice (ECPP) provides operationalization for the main known somatization factors and psychosomatic syndromes.
3. The Psychosomatic Arc offers the *systematic description and visualization of the somatization process* to optimize its understanding, treatment planning, and facilitation.
4. The five-stage psychotherapeutic strategy provides a systematic framework to effectively and sequentially address every critical point of somatization:

   Stage one: Observation *(to engage the patient in therapy and to reorganize the patient's perception)*.

   Stage two: Inventory *(to differentiate the contents and dynamics of the inner conflict underlying the psychosomatic symptoms)*.

   Stage three: Stimulation (Situational Encouragement) *(to find resources of self-trust, conditioning the inspiration and curiosity to experiment)*.

   Stage four: Verbalization *(to optimize the resolution of the key conflict)*.

   Stage five: Broadening *(to develop capacities supporting a healthy, balanced life after therapy)*.

Key points

1. Positive Psychosomatics based on positive psychotherapy addresses symptoms as a capacity to physically discharge the tension of the underlying inner conflicts.
2. The Evaluation Criteria for Psychosomatic Practice (ECPP) provides operationalization for the main known somatization factors and psychosomatic syndromes.
3. The psychosomatic arc offers the *systematic description and visualization of the somatization process*.
4. The five-stage psychotherapeutic strategy addresses every critical point of somatization systematically, sequentially, and effectively.

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16 Positive Child and Adolescent Psychotherapy

Roman Ciesielski

Introduction

The Fate of a Butterfly

A man discovered a butterfly cocoon and one day noticed a small hole appearing. He watched attentively for several hours as the butterfly struggled to squeeze through the tiny opening. Eventually, it seemed to be stuck and unable to progress. Out of compassion, the man decided to help. He used a pair of scissors to cut away the remaining part of the cocoon, allowing the butterfly to emerge easily. However, the butterfly had a swollen body and small, crumpled wings. The man didn't worry and expected the wings to expand and strengthen for flight. But this never happened. The butterfly spent its life crawling with small wings and a bloated body. The man, though well-intentioned, didn't realize that the butterfly's struggle to escape the cocoon was necessary to force fluid from its body into its wings, preparing it for flight once it emerged.

Nossrat Peseschkian's humanistic concept emphasizes the development of innate capacities from birth through an individual's life. This process is most intense in early childhood when self-image and world perception are formed. Peseschkian [1] stressed the importance of models describing a child's developmental needs, highlighting their delicacy and sensitivity, and defining optimal growth and maturation conditions. Understanding developmental norms and recognizing abnormalities are crucial in child and adolescent psychotherapy. Another key aspect discussed is the family and environmental context in which a child grows. A therapist's ability to include the family-social dimension in diagnosis and therapy is vital for the child's development.

Developmental Perspective

Understanding the complexities of adolescence benefits from proven developmental models. Erik Erikson's [2] psychodynamic model of identity development is valuable for individual development, while Evelyn Duvall's model of family development cycles [3] is essential for understanding the family environment.

Psychosocial Development (Erik Erikson)

Erikson (1997) identifies normative developmental crises. Successful resolution equips a child with new psychological and social skills, facilitating their transition to subsequent life stages. Unresolved crises can cause fixation and

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_16

psychological conflicts, such as mistrust, inferiority, or shame, that resurface later.

Family Development Cycles (Evelyn Duvall)

Duvall's model [3] suggests that families face similar challenges at different stages, requiring specific resources and adaptive capacities. Without these resources, the family system may react defensively, causing the child to regress developmentally. Examples include separation anxiety at kindergarten or adolescent depression at high school onset.

Development and Differentiation of Actual Capabilities (N. Peseschkian)

Peseschkian [4] describes the phases of a child's ego maturation within the family-social system, identifying two significant stages:

- *Stage* 1: Formation of primary capabilities (up to about age 2)
- *Stage* 2: Formation of secondary capabilities (above age 2)

Primary Capabilities and Attachment

John Bowlby's attachment theory [5, 6], later extended by Mary Ainsworth [7, 8], emphasizes the importance of the attachment relationship between a child and their primary caregiver, usually the mother. This bond influences the child's proprioceptive, emotional, cognitive, and social development and serves as a model for future close relationships. A secure attachment style fosters optimal development and basic trust in oneself and the environment. Bowlby also described non-secure attachment styles, where the caregiver fails to provide adequate care (anxiety-ambivalent, avoidant, and disorganized styles). Such deprivation can lead to long-term social dysfunctions and psychological disorders.

Peseschkian used the language of primary capabilities to describe attachment relationships. The primary caregiver provides relational experiences necessary for growth, such as physical and emotional contact, trust, and patience. Deprivation of these needs impedes proper personality development [9].

Secondary Capabilities and Social Education

In many cases after age 2, a child increasingly encounters social influences. The circle of role models expands, and societal expectations of behavior become clearer. Positive reinforcement encourages desirable behavior, while negative reinforcement discourages inappropriate behavior. Peseschkian [10, 11] defines these learned behaviors as secondary capabilities (e.g., conscientiousness, politeness, punctuality, etc.). These capabilities help the child adapt to societal norms but may conflict with their inner desires.

Examples

- *Don't say everything that comes to mind; consider what others will think.*
- *Don't eat all the candy; remember your siblings also like candy.*
- *Show patience; don't be so hasty.*

Positive reinforcements also play a role:

- *If you do your homework, Daddy will take you for ice cream.*
- *If you behave well at the table, you can watch a fairy tale.*
- *If you clean your room, we'll plan a bike ride around town.*

The Development of Social Skills and the Formation of Conflict

Peseschkian referred to these adaptive patterns as basic conflicts, indicating their potential to cause internal conflict. These conflicts arise when compromises are over-generalized as universal

strategies. Whether these compromises lead to internal conflict depends on various factors, including individual personality predispositions.

Traits and Underlying Conflicts

Certain traits like meticulousness, thoroughness, perfectionism, submissiveness, and shyness can make a child vulnerable to developing and maintaining specific underlying conflicts.

Examples

- *When I do something almost perfectly, my parents praise me, and I feel proud of myself.*
- *By not mouthing off like my peers, my mother shows me more tenderness and patience.*

Family Norms and Coping Strategies

Every family has its own explicit and implicit norms and values, shaping its cultural heritage. The impact of these norms depends on the family's adherence to traditions and tolerance for differences.

Examples

- *Dirt is washed at home, so don't you dare talk to anyone about what goes on in our family.* (rigidity)
- *We know you like to share with friends what is discussed at home. At least try not to reveal family secrets that could put us in a bad light.* (flexibility)

Openness to Social Experiences

The family's tolerance for differences and willingness to learn from social exchanges plays a crucial role in a child's psychological development. A friendly, accepting attitude reduces the risk of internal conflicts, whereas excessive hermeticism and discriminatory attitudes increase this risk.

Examples

- *They are not worth dealing with. They plot against us and gossip. Stop going to them.* (discriminatory attitude)
- *It's interesting that you notice differences in how we handle trouble and how Peter's family does. Let's discuss it; maybe we can learn something new.* (tolerant attitude)

Cooperation with External Systems

School, peer groups, and other external environments provide valuable educational and social experiences, often different from those at home. The child's psychological comfort depends on the family's ability to balance these differences.

Examples

- *In this school, they teach nothing but cleverness and cunning. They put all the responsibility on us. Again, we'll have to force them to care about education.* (antagonistic attitude)
- *School teaches some things, and family teaches others. We complement each other, and your education is more effective. It's good we can talk about your progress with your tutor.* (cooperative attitude)

Adapted compromises can enrich a child's social skills if varied, applied flexibly, and open to social exchange.

Internal and External Factors Influencing Development

Whether a child's internal compromises foster development or conflict depends on how their family is positioned on each of two continua:

- Openness–closeness
- Flexibility–rigidity

Socio-Environmental Context

The relationship between a child's psychodynamics and family sociodynamics is crucial. However, the child's functioning in non-family environments (education, sports, recreation) also plays a significant role. These settings can reveal different abilities and skills, providing alternative psychic resources and influencing self-esteem. Conversely, they may also expose difficulties and limitations not visible in the family context.

Gathering Information for Diagnosis

Positive therapists should gather information from all available sources during diagnosis. Inconsistent data warrants a joint diagnostic-consultation meeting with those providing conflicting information. When caregivers are knowledgeable and proactive about seeking specialized consultation, it aids the diagnostician. However, if caregivers deny or fail to recognize emotional difficulties in a family, reports often come from schools or other systems like social welfare.

Cooperation with Caregivers

Regardless of the report's source, cooperation with caregivers is essential. They should be viewed as allies, with restraint in judgment. Initial communication difficulties often stem from fear, anxiety, worry, or guilt.

The Therapeutic Relationship and Modeling Process

The therapeutic relationship is fundamental in working with children, fostering secure attachment with the therapist, and providing corrective emotional experiences [12, 13]. The therapist uses knowledge from interviews, observations, and interactions to identify and develop latent capabilities like trust, hope, and acceptance. The therapist can model these capabilities, creating favorable conditions for the child's ego development.

Involving the Family

Since the child is primarily raised by family and significant others, the process should also include adults from the primary support group. Therapeutic interactions often aim to provide models for adults to communicate and build relationships with the child. Activities in therapy and home assignments can strengthen parental competence, enabling a broader repertoire of relational role models.

The Therapeutic Contract

The therapeutic contract for minors differs from adults. Parents must be informed and agree to counseling and treatment, except in cases of violence or loss of parental rights. Parents should support their child's psychotherapy and have access to general information about therapy goals and methods. The extent of sensitive data shared depends on the child's age, and confidentiality principles should be discussed with the child and family.

Confidentiality and Risk Management

Confidentiality does not apply when the child's life or the health and/or life of others are at risk. In such cases, the psychotherapist must take preventive measures. It's important to agree on confidentiality issues regarding addictions, self-harm, self-starvation, or other risky behaviors. Addressing confidentiality in three-way communication requires ethical sensitivity and clinical experience.

Contracting with Children and Families

The therapeutic contract varies depending on the child's age. For adolescents, there's more direct involvement in contracting therapy. For younger children, the family becomes the contracting party. The contract should include:

- Establishing the diagnosis
- Determining treatment goals
- Preliminary agreement on therapy duration (possibly the number of sessions)
- Rules of contact with the family
- Extent of information provided to the family
- General rules of confidentiality
- Form of payment

Types of Psychotherapy During Development

Therapeutic work with children and adolescents can include:

- Individual therapy
- Individual therapy with parental or family consultations
- Family therapy
- Group therapy

The appropriate form of psychotherapy depends on the child's age, level of dysfunction, and agreements with the child and parents. Generally, the younger the child, the more advisable parental participation in therapy. For adolescents, therapy may be individual or include occasional family consultations to discuss progress or priorities. Discussions with parents should preferably occur in the adolescent's presence to avoid creating an implicit alliance against the child.

Indications for Family Therapy

Family therapy may be more suitable in situations like:

- The child's reluctance to participate in therapy
- Developmental, intellectual, or cognitive limitations
- Disorders requiring close family cooperation (e.g., schizophrenia, ASD, eating disorders)
- Prevalence of family relationship issues
- Ineffectiveness of previous therapies
- Specific recommendations from other specialists familiar with the child's situation

Group Psychotherapy

Group psychotherapy is particularly recommended during the developmental period. It can be the main or complementary method of treatment. It allows peers to exchange experiences, acting as a social mirror and helping to normalize emotional and relational difficulties.

Therapeutic Techniques in Developmental Period

Therapeutic work with children and adolescents requires flexible adaptation of tools and techniques based on the patient's age and maturity [14]. Younger children benefit more from creative play and parental involvement, while adolescents engage in activities similar to those used with adults, with less direct parental participation. Therapeutic methods aim to improve:

- Communicative-expressive skills
- Cognitive functions (memory, attention, perception)
- Sensory-perceptual abilities
- Emotional regulation
- Relational-social skills
- Self-esteem and self-concept
- Coping and problem-solving strategies

Therapists select techniques based on the child's abilities and previously recognized skills, providing positive reinforcement for what the child already knows and can do.

Common Techniques in PPT

- Non-directive communication (metaphorical dialogues, stories, fables, story arranging, and writing)
- Psycho-sketching, sculpting, modeling, and constructing
- Thematic games and cards (learning about emotions, self-discovery, projective methods)
- Interactive play (puppets, dolls, Lego figures)
- Figure boards
- Psychodrama, role-play, role-swapping, symptom interview, etc.

Structure of the Therapeutic Process

The therapeutic process in PPT is divided into five strategic stages, creating a clear reference frame for both the patient and therapist. This structure helps monitor therapy effects and revise initial assumptions. The metaphor of a road symbolizes the healing process, appealing to children who understand the journey's beginning, end, and the idea of traveling with companions, using signposts, and facing obstacles.

Stage I: Observation and Distancing
The first step in the five-stage process of PPT involves making an initial diagnosis of the child's issues and the family system. The positive therapist gathers information from various sources, such as caregivers, extended family, educators, and psychologists, to form a coherent overall picture. Observing the child in their natural interactions with significant others is extremely valuable. Therefore, accompanying persons are invited to the initial consultation, and the child is encouraged to react and speak freely. This helps to understand the customs in the child's environment, what the family condones, and what is taboo. Often, the power dynamics and structure of the family system are recognized during the first meeting.

During this phase, we aim to understand the context of the refferal, i.e., the specifics of the current conflict and its associated difficulties, symptoms, and suffering. If one narrative is overwhelming or dominant, it's important to restore balance.

Examples
- *Sophie, you have been silent so far, but we've noticed you listening attentively to your parents. We'd like to hear from you now. Could you describe what worries you most at the moment?*
- *John, see this teddy bear with a sad face? You can hold it and imagine how it feels when it's alone and its parents are busy at work. Can you tell us how it feels?*

The observation phase also helps to view past problems from a bird's eye view (metaposition).

Examples
- *How does your eldest son, who isn't here today, describe these difficulties? How does his understanding of Sonya's situation differ?*
- *Betty, imagine talking to your best friend Dorothea, who knows your deepest thoughts and feelings. What would she share about you if she were here?*

Understanding the function of symptoms and positively reformulating them is valuable for future cooperation with the child and their caregivers.

Example
- *Dominica, have you noticed that since your anxiety stopped you from going to school and speaking up, your parents have been secretly discussing how to help you overcome it? Would you like to join their discussions and maybe give them some advice?*

In conclusion, this first stage allows for a preliminary diagnosis of the child's situation, recognizing both the child's and the family's capabilities and limitations. It also helps to identify the current conflict and the coping methods used. It's useful to determine who will participate in therapy and how, and what the therapeutic contract will include.

Stage II: Inventory

The second step focuses more on the past and its relation to the present. We explore the origins of conflicts and associated symptoms and dysfunctions with questions like: Since when? Under what circumstances? Who was involved? It's important for everyone to tell their story in their own words, and for loved ones to hear it. Despite the subjectivity and potential inconsistencies of these narratives, it's essential not to judge the narrator but to deepen understanding and open up discussions for different hypotheses to explain the genesis of problems. Positive therapists use tools like modeling dimensions or the family tree to delve into family core beliefs and traditions that may underlie a child's core conflicts. Basic conflicts (internalized compromises) can cause discomfort and suffering for the child, often serving as adaptive strategies within the family environment.

Example

- *Always speak your mind and we'll support you.* (Basic conflict pattern: Honesty equals family acceptance and support).

However, this family adaptation pattern might not apply in every situation, like with a teacher as an authority figure, where it could be seen as bad behavior, leading to criticism or rejection.

Example:

- *Always be polite and obedient, and we'll take care of you if needed.* (Basic conflict pattern: Politeness equals parental love and care).

This attitude might not always benefit the child in environments like a classroom or sports club, raising questions about parental intervention and support. The inventory phase involves exploring and recognizing all useful coping strategies the child and family have used previously, balancing problems and difficulties and utilizing them therapeutically.

In conclusion, the inventory stage deepens and expands knowledge of difficulties and coping strategies. The therapist takes an inquiring, exploratory approach rather than commenting or interpreting.

Stage III: Situational Encouragement

During this support phase, we focus on identifying and summarizing the child's mental, emotional, relational resources, skills, and abilities. This helps balance the child's self-esteem and counteract the negative effects of critical judgments from their environment. Often, in family or school settings, adults overly focus on the child's dysfunctions and inadequacies. For instance, a child diagnosed with ADHD may be labeled as hyperactive, inattentive, and restless. Repeated reprimands like *Sit still, Stop fidgeting,* or *Stop daydreaming* can severely diminish the child's self-esteem, making them feel perpetually criticized regardless of their actions. This often leads to worsening behavior and potential social exclusion. At PPT, we aim to counteract negative reinforcement early. We find it beneficial to meet with parents, family members, educators, and teachers to gather diverse insights into the child's behavior.

At this stage, we also reframe symptoms that may have previously been seen as acts of defiance or ill will. For example, a child's impulsive behavior may be linked to parental conflict, which can be viewed as the child's attempt to prevent family breakdown. Changing perceptions of previously negative situations can be achieved through various therapeutic methods, including open discussions with family members. Differing views and interpretations of events are explored, and the therapist facilitates constructive dialogue and compromise. Home-based activities can support this process.

Example

- *T: Simon, could you finish, as an exercise of your imagination, the story which starts as follows: 'Jacek was a very impatient boy and often spoke out of turn. This caused him many problems until he spent the summer at his grandfather's house, where he discovered new things about himself...'*

Therapeutic practice shows that children are interested in stories where the main character finds new strengths in seemingly hopeless situations. In the situational encouragement phase, we not only identify the child's psychological resources and reframe their symptoms positively

but also recognize the child's real limitations and those of their environment. We consider their neurodevelopmental, cognitive, and intellectual capabilities, as well as the maturity and competence of their caregivers. If needed, we expand interventions through additional support like speech therapy, pedagogy, neurology, or marriage counseling.

In conclusion, the third step in child and adolescent psychotherapy helps identify what is functioning well and what needs change, as well as sources of strength, motivation, and areas for further learning.

Step IV: Verbalization

The fourth step involves working through conflicts, which means understanding their internal dynamics (psychodynamics) and the family system's dynamics (sociodynamics). In earlier stages, we learned about the context of conflicts and their effects. Now, we motivate the child to gain insight into the nature of conflicts and their relation to family and interpersonal relationships.

Clinical Illustration

During a session, 14-year-old Sophia realized that need for acceptance and recognition led her to develop strong accuracy and conscientiousness. However, her perfectionism caused her to lose spontaneity and joy. This realization prompted her to explore how she could regain enjoyment in life and find other ways to feel important and accepted. Sophia's desire for recognition and high self-esteem made her almost perfect, but she still wanted to feel loved regardless of her responsibilities and to have the right to make her own mistakes. These desires were verbalized during her therapy. The verbalization phase allows for trial and error, where, with therapist and family support, the child can test their ideas and learn new skills that improve self-esteem and social image. Sophia joined a dance club, discovering her passion and gaining appreciation from her coaches and peers.

Changing limiting thought and behavior patterns helps unlock hidden resources and develop more effective problem-solving strategies.

Clinical Illustration

Johnny, a 13-year-old boy, began self-harming, which disturbed his family and teacher. His scars seemed to express his suffering, and he was reluctant to explain his behavior. It appeared he was punishing his family without a clear reason. Through therapy, Johnny revealed his difficulty managing sudden strong emotions of anger and grief, linked to his best friend's emigration. He felt unloved compared to his older brothers, wath was intensyfing his distress even more. His therapist helped him understand his inner conflict about love and justice. Imagining Mrs. Justice, he voiced accusations against family members, with the therapist acting as their advocate. Johnny eventually recognized some feelings were subjective and learned about his family's different expressions of love. Speaking at a family reunion, he invoked his late grandmother's spirit to articulate his understanding of justice, leading to the cessation of his self-harming behavior.

Step V: Expanding the Goals

The final step prepares the child to function independently of the therapist. The child needs to recognize their strengths and skills acquired during therapy and know who to turn to when needed. Parents should be involved in reinforcing positive changes and serving as primary support. Discussing potential future scenarios and coping strategies is also useful.

Example

- *Michael, you're imaginative and aware of your strengths. See that basket with various objects? Let's take a trip to the future in a spaceship where you're the captain. Choose five items from the basket that remind you of your strengths for this journey. Explain why*

you chose each item and what it means to you. We'll then discuss potential challenges in space and the help you might need.

Conclusion

Child and adolescent psychotherapy in the PPT approach shares similarities with adult psychotherapy in explaining the etiology of disorders and the mechanisms of their formation and maintenance. Both use the microtrauma and conflict theory of psychodynamic therapy and tools like the Balance Model and the Four Dimensions of Modeling. However, differences lie in understanding developmental regularities and integrating cooperation with the child's family and other significant systems. The interventions and therapeutic tools must be age-appropriate and developmentally suitable.

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Positive Psychotherapy and Autism Spectrum Disorders

17

Ewa Dobiała

Introduction

As an integral part of society, autistic people often encounter barriers in understanding their unique needs, both in everyday life and in therapeutic contexts. The diverse developmental pattern of these people requires the medical and psychological community to constantly adapt and expand knowledge about the specificity and diversity of functioning on the autism spectrum. Despite rapid progress in autism research, there is a lack of uniform, effective therapeutic methods, especially for adults. This problem is getting worse every year as the number of diagnosed people increases, particularly among women, who require specialized knowledge due to more subtle manifestations of autism. An increase in the number of people aware of the neurodevelopmental differences among those living with autism should be expected due to increasing diagnostic possibilities, greater public access to current medical knowledge, numerous co-occurring and often secondary psychosomatic, somatic, emotional and mental disorders, and the hereditary nature of autistic development.

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Medical, Psychological, and Sociocultural Aspects of Autism

Medical Aspects of Autism

The modern understanding of the autism spectrum assumes its neurobiological nature, which is based on both genetic factors [9, 14, 16] and environmental ones. These factors influence brain development in the pre- and postnatal period. Research conducted so far has not confirmed any isolated etiological factors. Therefore, the autism spectrum is considered one of the most heterogeneous neuropsychiatric disorders. Numerous cohort studies have excluded the view initiated in 1998 linking autism with vaccinations [8].

Differences in over 700 genes have been confirmed in autistic people, in which de novo mutations are very rarely observed [15]. Despite the clear etiological role of genetic factors, their phenotypic expression in autistic people remains highly variable and modulated by numerous factors: prenatal, perinatal, and postnatal. There are studies showing some differences in the architecture of the cerebellum and limbic system in autistic people, as well as changes in the frontal and temporal cortex areas of the frontal lobe as well as in the formation of the cortical layer and differences in neuronal differentiation [7]. The phenomena described are usually subtle and do not currently provide grounds for use in the diagnostic process.

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_17

During the life course of a person on the autism spectrum, medical challenges include not only developmental differences in the cognitive, sensory, motor and communication areas, but also diseases and mental disorders of both primary and secondary nature. Their frequency increases the diversity of clinical images presented by autistic people.

Modern research shows that 50%of people with ASD have at least four confirmed conditions (psychiatric, neurological, endocrine, rheumatological, gastroenterological, etc.), and over 95% of children with autism have at least one additional diagnosis [1]. Clinically, 83% of children with ASD have an additional neurodevelopmental diagnosis, 10% have at least one psychiatric diagnosis (anxiety disorders, depressive disorders, eating disorders, etc.), and 16% have a neurological diagnosis.

In the population of adults on the autism spectrum, the incidence of diseases and disorders increases significantly. They are often secondary to microtraumas and traumas experienced during childhood and adolescence [2]. At least one additional psychiatric diagnosis is found in 50–70% of autistic adults. Clinically, the risk of depression is three times higher than in the non-autistic population, the risk of schizophrenia is 22 times higher, and the risk of suicide attempt is five times higher. Only 50% of autistic people who attempt suicide are diagnosed with depression. The occurrence of immunological, endocrine, and rheumatological disorders is much more common than in the general population. Single prospective population studies indicate that the mortality rate of people on the autism spectrum is twice as high as the general population. An additional factor that complicates treatment are differences in drug metabolism, observed clinically and increasingly confirmed in research among the autistic population. These differences affect the level of drug safety and the number of side effects [6].

Psychological Problems of Adults with Autism Spectrum Disorder

Psychological problems of people on the autism spectrum, especially in the dimension of relationships, are strongly related to congenital differences in sensory and motor development, which from the first days of life influence the shape of the emerging personality structure. In the approach of Positive Psychotherapy (PPT), it is assumed that we are born with two fundamental capacities: the ability to love and the ability to learn. These abilities constitute the foundation for the emerging primary and secondary capacities, which are the key to understanding intrapsychic dynamics in the sense of PPT (Table 17.1).

Development of Primary Capacities in Autistic People

The development of primary capacities can be described in four key areas regarding: body, contact, productivity, and imagination [11]. In autistic people, each of these areas develops in a unique way, mainly due to their different experiences resulting from central sensory processing disorders and differences in motor development (Table 17.2).

Body Area—Contact, Pleasure

Autistic people experience sensory discomfort from the first days, which affects the develop-

Table 17.1 Primary and secondary capacities

Primary capacities [13]	Primary capacities Adapted by I. Kirillov [10]	Secondary capacities [13]
Love	Contact	Reliability
Trust	Pleasures	Obedience
Time	Love	Justice
Contact	Care	Orderliness
Unity	Trust	Cleanliness
Faith	Time	Punctuality
Hope	Meaning	Courtesy
Sexuality	Idealization	Accuracy
Confidence		Diligence
The ability to doubt		Openness
Confidence		Politeness
Modeling		Faithfulness
Patience		Thrift

Table 17.2 Basic potentialities and their function Kirillov [10]

Capacity	Development area	Function
Contact	Body	Contact with yourself Contact with the object
Pleasure	Body	Recognizing your own emotions Pleasure from one's own physicality Recognizing others' emotions Pleasure in the physicality of the object
Love	Contact	Ability to love yourself Ability to create emotional bonds Ability to cope with separation
Care	Contact	Ability to take care of yourself Ability to ask for and accept help Ability to care for others Ability to protect others from your own impulses
Time	Productivity	Ability to recognize one's own circadian rhythms Ability to create your own circadian rhythms Ability to recognize the circadian rhythms of others Ability to share circadian rhythms
Trust	Productivity	Ability to trust yourself Ability to understand and manage your own impulses Ability to trust others Ability to manage relationships
Meaning	Imagination	Ability to understand causes Ability to manage one's causes to understand the causality of others Ability to share ideas with others
Idealization	Imagination	Ability to idealize yourself Ability to deidealize yourself Ability to idealize others Ability to deidealize others

ment of capacities in the body area such as contact and pleasure. Difficulties concern both the ability to stay in contact, pleasure with oneself, receiving physical contact and pleasure from others, as well as sharing contact and pleasure with another person in order to build an emotional bond.

Contact Area—Love, Care

According to the phases development of Margaret Mahler, in about two months the infant passes from the autistic phase to the symbiotic phase, in which he begins to notice an object.

This is the moment when the capacities related to contact (love, care) begin to develop. Caregivers' lack of awareness of the specific sensory needs of autistic infants most often hinders the development of these abilities. Sensory properties such as physical proximity, timbre and intensity of voice, and touch, which are key to experiencing care and love in neurotypical infants, become the cause of difficulties in emotional development in this area, making the personality structure of autistic people more fragile, and narrowing the emerging window of tolerance for stress.

Area of Productivity—Trust, Time

Already in the symbiosis phase (2–6 months of age), as well as in the phase of separation and individuation (6–24 months), the capacities of trust and time begin to develop in the relationship with the object. Hypo- and hyper-variance of motor patterns, later often described clinically as dyspraxia, hinder the development of trust. Due to the difficult development of primary capacities in the area of body and contact, synchronization of circadian rhythms in autistic people occurs later, often being a source of clinical difficulties, e.g., sleep-wake rhythm disturbances.

The Area of Imagination: Meaning, Idealization

The area of imagination, which includes the development of the capacities of meaning and idealization used to regulate internalized symbolic structures, concepts, and narratives, may become an area that helps stabilize the personality structure in a group of autistic people within the intellectual norm. Development in imaginative capacities is crucial to how autistic people meet their emotional needs and attach to patterns, structures, and passions. Often, imaginative capacities are the only available methods of emotional regulation and management of the effects of current conflicts.

Secondary Capacities and Their Development

The development of secondary capacities in autistic people also has a certain specificity. This results from difficulties in understanding the social context and significant difficulties in experiencing a sense of security in relationships with other people [2]. In autistic people, we often observe their rigid development and subsequent compliance in a very strict manner, which often leads to difficulties in flexible adaptation to changing social circumstances and, secondarily, to the activation of numerous interpersonal conflicts. In the concept of positive psychotherapy, this can be understood through the prism of frequent activation of conflicts, which may open space for both internal conflict and the dynamics of microtrauma.

Balance Model in Autistic People

The balance model is the way in which each person distributes his or her daily life activity in four main areas (Fig. 17.1):

1. Body area.
2. Productivity area (other names for the area: work/achievements).
3. Contact area.
4. Imagination area (other names for the area: creativity/spirituality).

The balance between the energy put into each of these spaces in life constitutes the individual pattern of a given person, biologically, psychodynamically, systemically, and transculturally conditioned. Reflecting on the changes occurring in the balance model in response to the patient's current conflict may be a source of both functional, psychodynamic, and systemic diagnosis of the individual. For further details on the balance model, see Chaps. 2 and 8.

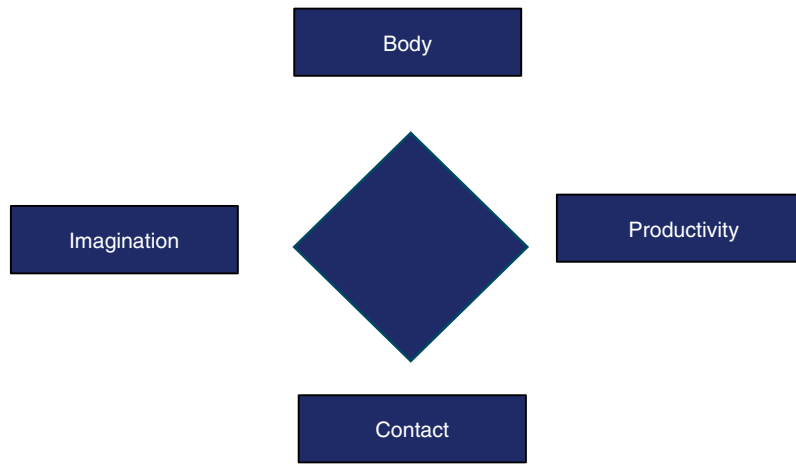


Fig. 17.1 Balance model [13]

Specificity of the Balance Model in Autistic People

1. Body Area: Autistic people often experience a variety of sensory and motor challenges. Central sensory processing disorders and differences in motor development affect their functioning in the body area. These differences also affect sexual development [3] and the secondary occurrence of psychosomatic and autoimmune diseases.
2. Productivity Area: This area refers to work and achievements that may be challenging for autistic people, especially in traditional, non-adaptive work environments. People on the autism spectrum often demonstrate exceptional skills in detailed analysis or intense concentration, but may have difficulty with multitasking or interpersonal communication.
3. Area of contact: Social interactions are often one of the greatest challenges for autistic people due to frequent difficulties in interpreting the non-verbal signals of neurotypical people or initiating and maintaining conversations. An additional problem is the neurotypical world's lack of understanding of the specificity of autistic communication and the phenomenon of double empathy. In girls and women, the difficulties described above may be masked by a higher level of socialization [4].
4. Imagination Area: Creativity and spirituality are areas that can offer autistic people a significant source of self-expression and coping with emotions. Some autistic people may demonstrate exceptional artistic or scientific abilities, using their unique ways of processing information.
5. The balance model of autistic people expressed in hours (daily distribution in four areas using a 24-h day) may differ from the energy model: a daily schedule in four areas using 100% of the energy to be used during the 24-h period. This is related to the specificity of development, which makes certain activities much more energy-consuming than others. For example, an hour spent in a peer group can consume a similar amount of energy as 10 h spent in an activity related to one's passion. It is important for autistic people to identify their own specific balance model so that they can best allocate their energy resources in these four areas. Imbalances, such as sensory or social overload, can lead to stress, information overload, adjustment difficulties, and other emotional and somatic symptoms.

The Socio-cultural Dimension of Autism

The socio-cultural dimension of autism is a research area that draws attention to the complexity of the lives of autistic people in the social and cultural context, focusing on questions about autistic identity, which become particularly important in the light of modern social and cultural changes. Over the years, the growing awareness and understanding of autism as a neurological diversity has influenced the way society perceives autistic people, as well as the way they identify themselves and interpret their place in the world [5].

Autistic Identity in Social Context

Autistic identity is a relatively new concept that has gained importance with the development of the neurodiversity and autistic rights movements. People on the autism spectrum may perceive themselves through the prism of their own unique cognitive and social experiences, which differ from those of neurotypical people. Modern Western societies, on the one hand, create space for individualization and acceptance of diversity, but on the other hand, they still consciously or unconsciously build barriers and challenges, such as stigmatization or marginalization of people with neurological differences.

The Double Empathy Phenomenon

The concept of "double empathy", developed by autism researchers [12], describes difficulties in

mutual understanding between autistic and neurotypical people. This problem results not only from differences in the way social signals are processed, but also from a lack of mutual emotional and cognitive understanding. Solving this problem requires greater knowledge and openness on the part of neurotypical people, as well as better communication on the part of autistic people, which is key to building a more inclusive society. Research shows that it is much more difficult for neurotypical people to understand the emotional situation of autistic people than the other way around.

Psychotherapy for Individuals on the Autism Spectrum

In terms of Positive Psychotherapy, we distinguish five stages of work: observation, inventory, situational encouragement, verbalization, and broadening the goals.

Stage of Observation

Observation allows us to recognize what kind of problem is reported by the patient. It opens a space for diagnostic mindfulness, reflection on the emerging dynamics of transference and countertransference, and development of a contract with the patient (Table 17.3).

Stage of Inventory

This marks the time to therapeutically look at the patient's lifeline. At this stage, the patient's current traumas and microtraumas, the systemic context, the balance model, the modeling dimension, the relational dimension, active capacities, and strategies for dealing with current conflicts are identified. Therapeutic work with an autistic person at this stage requires knowledge of the developmental specificity of autistic people (Table 17.4).

Stage of Situational Encouragement

This is a stage very consciously emphasized in the Positive Psychotherapy approach. The "insight" work that takes place later also involves the strengthening of those parts of the patient where he has contact with his resources and has developed survival strategies for dealing with traumatic experience triggers. This prevents possible retraumatization and also initiates the process of conscious contact with salutogenetic resources and strategies. At the stage of situational encouragement, it is important to draw the patient's attention to the "positum" of the situation. This allows us to see both the pain and suffering the patient brings, as well as the resources he has managed to develop thanks to the trauma experience (Table 17.5).

Table 17.3 Objectives and tools of the observation stage—own work

Stage of the therapeutic process	Goals	Sample PPT tools	Specificity of working with an autistic person
1. Observation	1. Determining the current conflict, reaction in the area of the balance model, and the key conflict 2. Reflection on the phenomena of transference and countertransference 3. Therapeutic contract	1. Observation 2. Transference 3. Countertransference 4. Current conflict 5. Key conflict 6. Balance model 7. Secondary capacities 8. Fairy tales, metaphors 9. Therapeutic contract	1. Understanding the specific difficulties of an autistic person 2. Recognizing unique abilities 3. Reflection on the dynamics of the phenomenon of double empathy 4. Differential diagnosis 5. Adapting the contract to an autistic person (transparency, understandability, structure, sensory safety of the therapeutic space) 6. Empathy and individual approach 7. Therapist's stability and clarity of communication

Table 17.4 Goals and tools of the Inventory stage—own work

Stage of the therapeutic process	Goals	Sample PPT tools	Specificity of working with an autistic person
2. Inventory	1. Recognizing the basic and inner conflict 2. Recognition of past traumas and microtraumas in life 3. Work on personality structure (integration, development of primary capacities)	1. Development of primary capacities 2. Development of secondary capacities 3. Active capacities 4. Basic and inner conflict 5. Balance model 6. Differential analysis inventory 7. WIPPF 8. The tree of life 9. Lifeline 10. Family messages 11. Modeling, relational, and responsibility dimensions 12. Fairy tales, metaphors	1. Understanding developmental differences in the area of primary and secondary capacities 2. Readiness to work in the area of trauma (stabilization, reconsolidation, integration) 3. Awareness of the hereditary nature of autism when working with a genogram

Table 17.5 Goals and tools of the situational reinforcement stage—own work

Stage of the therapeutic process	Goals	Sample PPT tools	Specificity of working with an autistic person
4. Situational encouragement	1. Association with resources 2. Strengthening the healthy part of the ego 3. Activation of salutogenetic factors	1. Positum 2. Fairy tales, metaphors, anecdotes	1. Understanding the regulating role of passion in the life of an autistic person. 2. Understanding the regulating role of stereotypical behaviors and stims of an autistic person.

Stage of Verbalization

This is a time of in-depth reflection by the patient on his intrapsychic dynamics. By recognizing the current conflict, and understanding the content of active capacities and the structure of the basic conflict, space opens for making intrapsychic dynamics more flexible, recognizing the basic conflict, and solving possible neurotic fixation.

Positive psychotherapy conceptualizes work with patients beyond the neurotic level of personality organization. In lower-integrated patients, using the modern understanding of operationalized psychodynamic diagnosis, clinical therapeutic work includes the areas of: self-perception, impulse control, contact with one's own and others' emotions (e.g., mentalization), and the internal object. Conceptualization of work in such clinical situations is possible thanks to in-depth reflection on the stages of interaction occurring in therapeutic work (attachment-differentiation-detachment) (Table 17.6).

Stage of Broadening the Goals

This is the last element of the therapeutic process. It marks a time of ending the therapeutic relationship, shifting attention toward the future, and reflecting on the readiness to take responsibility in the dimension of "I", "you", "we", and "primary-we" (Table 17.7).

The described stages of therapeutic work interpenetrate throughout the entire duration of therapy. They describe both the entire process of interactions and the course of a single session.

Table 17.6 Goals and tools of the verbalization stage—own work

Stage of the therapeutic process	Goals	Sample PPT tools	Specificity of working with an autistic person
4. Verbalization	1. Finding healthier ways to meet emotional needs 2. Personality integration	1. Clarification 2. Confrontation 3. Interpretation 4. Fairy tales, metaphors, and anecdotes 5. Balance model.	1. Understanding the role of self-awareness 2. Understanding the role of autistic identity 3. Understanding autistic culture 4. Understanding the role of experience resulting from belonging to a minority group with a history of eugenics, discrimination, and stigmatization 5. Understanding sensory, motor, emotional, communication, social, and cultural differences 6. Understanding the differences in sexual development

Table 17.7 Goals and tools of the goal expansion stage—own work

Stage of the therapeutic process	Goals	Example PPT tools	Specificity of working with an autistic person
5. Broadening the goals	1. Orientation for future 2. Completion of the therapeutic process	1. Balance model. 2. Responsibility model. 3. Fairy tales, metaphors, and anecdotes.	1. Developing a method of implementing and monitoring progress. 2. The importance of possible follow-up and long-term support. 3. Long-term cooperation and building patterns.

The transparency of the structure of Positive Psychotherapy allows it to also be used in self-help work. At each stage of the therapeutic process, the psychotherapist's reflection concerns all dimensions (intrapsychic, interpersonal, systemic, and transcultural).

Summary

The therapeutic process for autistic adults is dynamic and requires flexibility and constant reflection on the part of both the therapist and the client. Success in therapy depends on a holistic approach that takes into account the unique needs of the autistic person, their strengths, challenges, and social environment. By focusing on understanding, support, and continuous development, therapy can lead to a significant improvement in the quality of life and greater independence for an autistic person.

Positive Psychotherapy as a meta-theory integrates the following dimensions: psychodynamic, cognitive-behavioral, systemic, transcultural, and humanistic in psychotherapeutic work, thereby finding its application in working with this group of patients. The clarity of the conflict model concept and understanding the level of personality integration allows working with autistic people with different clinical presentations.

Due to the dynamic development of knowledge about neuropsychiatry and neurodiversity, therapists should be open to continuous expansion of their knowledge and skills by participating in specialized training and self-education in the field of neurodevelopment, including specific sensory, motor, emotional, communication, cognitive, and social functions of autistic people.

Key Points

1. *Autism spectrum*: Autism spectrum (AS) is understood as a distinct neurodevelopmental pattern that affects approximately 2% of the population. Autistic people struggle with various challenges in sensory, motor, emotional, communication, and cognitive functioning, which affects their intrapsychic, interpersonal, family, and social functioning.
2. *Medical aspects of autism*: Autism is considered one of the most heterogeneous neuropsychiatric disorders, resulting from the complex interaction of genetic and environmental factors. Autistic people often experience numerous co-occurring disorders, which further complicates their clinical picture.
3. *Psychological challenges of adults on the autism spectrum*: Positive psychotherapy describes among autistic people the development of primary and secondary capacities, which develop differently in this population due to their unique experiences and sensory and motor differences.
4. *Balance model*: The balance model, which covers four areas of life (body, productivity, contact, and imagination), is crucial to understanding and managing the energy resources of autistic people. Disturbances in the areas of life described in this model may lead to sensory overload and, secondarily, to various mental and psychosomatic symptoms.
5. *The socio-cultural dimension of autism*: Autistic identity and the phenomenon of dual empathy are important sociocultural aspects that influence the lives of autistic people. Building greater awareness and understanding of these issues is key to a more inclusive society, as well as to effective therapy.
6. *Holistic therapeutic approach*: Success in autism therapy requires a holistic approach that takes into account the unique needs, strengths, and challenges of people on the autism spectrum. Transcultural positive psychotherapy offers a structure, tools, and understanding of the development of the personality structure of an autistic person, allowing for effective work with this group of patients.

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18

Chronic Pain—The Need for Simultaneous Bio-psycho-social Diagnostics with the Help of Positive Psychotherapy

Claudia Christ

Chronic Pain

The *most common psychological disorder with pain as the main symptom is persistent somatoform pain disorder* ICD 10: F. 45.4 [6]., which is associated with an "*unpleasant sensory and emotional experience*". According to the definition of the International Association for the Study of Pain (IASP) [7], this can be accompanied by "actual or potential tissue damage or resemble such damage". This category also includes visceral pain or "functional body pain", which originates from the internal organs, such as the gastrointestinal tract, bladder, uterus, etc. The sensory nerves in the internal organs have pain receptors, nociceptors, which transmit information from the inside of the body to the brain or spinal cord. This serves to warn us of an illness, injury or infection. We cannot see inside and so compression, stretching, haemorrhage, tearing or tiny inflammations caused by infectious organisms trigger damage signals. Visceral pain can vary in intensity and is usually not easy to localise because the mapping of pain from inside the body is not very detailed. When asked, patients describe visceral pain as dull, sharp, intermittent, superficial, deep-seated, pressing, etc. Visceral pain also tends to radiate to other areas, making the overall perception of pain more diffuse and unpleasant. Bladder pain can radiate to the peritoneum, heart pain to the arm. Visceral pain can also be accompanied by nausea, vomiting, palpitations, anxiety and sweating. It is therefore understandable that treatment, diagnosis and therapy pose a particular challenge, especially as psycho-social and biographical circumstances play a major role in pain perception and processing alongside somatic factors. Not an easy medical endeavour. Personality traits, illness behaviour, everyday stress, stressful life events, psychological comorbidities and the effects of psychotherapy are proven influencing factors on the development and course of visceral pain. Therefore, bio-psycho-social simultaneous diagnostics should be carried out from the outset in (visceral) pain syndromes [7, 11].

Much Ado About Nothing?—Many Complaints, Mostly "Without Findings"

Who doesn't recognise these patients from everyday medical and therapeutic practice who describe their complaints at length, demand maximum diagnosis and have high expectations of the medical system. They usually come with thick files, numerous preliminary examinations

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_18

without dramatic medical findings and countless failed attempts at treatment. They have consulted various medical specialists and have undergone outpatient, inpatient and possibly rehabilitative treatment programmes. Ultimately, they leave the doctors in doubt as to whether something has been overlooked, and so the "complaint-diagnosis-therapy attempt" cycle starts all over again. They are so-called "high-utilisers" and therefore cause above-average costs in the health-care system [1].

At some point, both the patient and the doctor are frustrated by the persistence of symptoms, or the diagnosis has been taken all the way to the "department for rare diseases". There are also feelings of helplessness, failure and anger towards the patient, "Now I've tried so hard as a doctor, read extra specialist books and everything doesn't help". More of the same does not seem to be the right therapeutic approach here, but which approach is the right one?

Patients with Somatoform Disorders Are Characterised by a Number of Distinctive Features

- Presence of many general symptoms over a very long period of time (alarm signals, sleep disorders and vegetative complaints).
- Diffuse complaints that cannot be described precisely.
- Discrepancy between subjective complaints and objective conventional medical findings.
- Demanding attitude and high expectations of the medical system ("save me").
- Frequent changes of doctor and thick files ("luminary killer").
- Psycho-social aspects are denied or trivialised ("I'm not psycho").

Modern Understanding of Pain—"Result of Neuronal Networks"

From a historical perspective, three concepts of pain are distinguished, firstly as an "otherworldly threat", secondly as an "indication of disturbed body mechanics" [17] and thirdly as the "result of a multitude of neuronal networks". Before Descartes, pain was considered an "otherworldly threat" as "hellish torment". The English word "pain" comes from the Latin word "poena" = punishment. Since Descartes' Enlightenment, pain has been seen as a "damage indicator for physical processes" as an "indication of disturbed body mechanics" [1]. However, if there was no demonstrable damage or the manifestations of pain did not match the findings, medicine found itself in need of an explanation. With the diagnosis of "psychalgia" (1967 in the F chapter of the ICD-8, International Classification of Diseases of the WHO [23]), this discrepancy seemed to be resolved; pain without findings remained a mental disorder and was later renamed from "psychogenic" to "somatoform" in the revisions of the ICD. At the same time, methods were developed in science to measure and understand pain more precisely, patients estimate on scales from 0–10 and adjective lists which help to characterise the quality of pain such as "dull", "stabbing", "shooting", "excruciating", etc. became standard.

However, it was not until 1970 that the term "chronic pain" found its way into medical literature. There was a lack of plausible causes and effective treatment. As a patient, you were quickly suspected of being a malingerer, a person who was having a good time at the expense of the community, or even mentally disturbed. "The concentrated power of classical medicine decided that the majority of patients with chronic pain were organically 'all right', that it was a programming error in mental processing that led to exaggerated pain. Psychologists were obliged to find this disorder." [20].

It is only in the latest revision of the ICD-11 that a broader understanding of pain is postulated. "Pain is an unpleasant sensory and emotional experience associated with or resembling actual or potential tissue damage. Chronic pain is pain that lasts longer than 3 months or recurs. Chronic pain is multifactorial: biological, psychological and social factors contribute to the pain syndrome" [7, 16, 23].

The concept of pain is also decoupled from nociception [15]. Pain is the result of comparing internal signals with contextual factors of one's own adaptability or resilience, work, social interaction, one's own biography and inner attitudes/concepts. Pain thus becomes a danger signaller, not a damage signaller. Nociception is a frequent but not necessary "input", pain is the "output" of our brain. Thus, the third and most modern concept of understanding pain as the "result of a multitude of networks in our brain" has found its way into science in the sense of a guardian function [12, 13].

Pain as a Guardian and Danger Signaller

Seeing "pain as a guardian" will bring about a paradigm shift for both patients and doctors in the future. Both will become "danger seekers" on a physical and socio-psychological level. The primary focus will not be on the disorder and therefore pathologisation, but on the danger. At this very moment, the neuronal networks are signalling danger or pain, "Attention, something is wrong here", "Attention, something here reminds me of a situation where something was wrong", "Attention, I am (wrongly) interpreting something here as danger". In contrast to nociception, pain is not genetically pre-installed, it is learned through experience and is therefore very individual, as we construct our own world. It can be very relieving for pain patients as well as for doctors and therapists in the future to systematically look for dangers in the first step and minimise them on various levels rather than immediately remaining trapped in the pathogenesis.

The vernacular is perhaps more advanced than science can currently prove. Phrases such as "I can't digest something", "There's a lot of weight on my shoulders", "I feel like vomiting" or "My abdomen is cramping up", "I'm pissed off" are clear indications of learned dangers in terms of visceral pain.

Patient Example

A 35-year-old patient, mother of two children (9 months and 4 years old, teacher on parental leave), has been suffering from recurrent abdominal pain for 4 years. She underwent several gastroenterological and gynaecological examinations. Endometriosis was ruled out, sonography, pelvic MRI and laboratory parameters were unremarkable. There was no indication of a tropical medical cause. When asked about stressful life events and stress, the patient reported that her husband was in bed with depression until midday and that she was responsible for the entire care of the small children with the derogatory remark "YOU wanted the children!". He doesn't do any work and spends a lot of time on the internet and watching porn. The family lives in the house of the husband's deceased mother. *The acute danger message is: Attention, something is wrong here, I am overwhelmed, left alone and devalued*. When asked, "Does such a pattern sound familiar to you?", she answers: "Yes, my mother was already depressed, and I had to be the sunshine and take on all the tasks from an early age". *The old danger message is: Attention, I know this pattern and it has always hurt me*. To my reaction "You have to expend a lot of energy every day and do everything on your own. I imagine that's very stressful. The derogatory attitude towards you also makes me angry. How do you feel about that? she cries and remarks "that was completely normal for me, I always blamed myself when someone was unhappy". *Danger message inner attitude: I am to blame when others are unhappy*.

Through a few verbal interventions, the patient was able to understand that the visceral pain has a deep sting of devaluation and role abuse. The body has been signalling "danger" for many years, which she is now able to assign well. The doctor's confidence in your resilience has strengthened you to initiate the separation from your husband. The feeling of powerlessness and guilt has given way to a creative energy. The word "psycho" was not used in the discus-

sions; an adjustment disorder was coded for the billing.

The Systematic "Search for Danger" in Chronic Pain Patients—Simultaneous Bio-psycho-social Diagnostics

In order to be able to carry out a pragmatic and systematic anamnesis of the individual "world of danger" in a short period of time in the tightly scheduled daily medical routine, a procedure based on five pillars makes sense.

1. Pillar of medical, organic and psychological diagnostics.
2. Pillar balance model of positive psychotherapy.
3. Pillar recording the stressors.
4. Pillar recording of life events.
5. Pillar inner attitude/beliefs.

The pillars are of equal value. The frequently used sentence "the organs are without findings; your pain is only psychological" prevents further simultaneous diagnosis and increases patients' fears of being psychologically labelled. It is more skilful to explain the simultaneous diagnosis of the five pillars as a standard procedure and to initially speak of pain as a "danger indicator". This opens up an understanding of how to deal with stressors and inner belief patterns. The five-pillar model also enables doctors to proceed systematically and not to avoid the psycho-social area as "incomprehensible terrain" (Fig. 18.1).

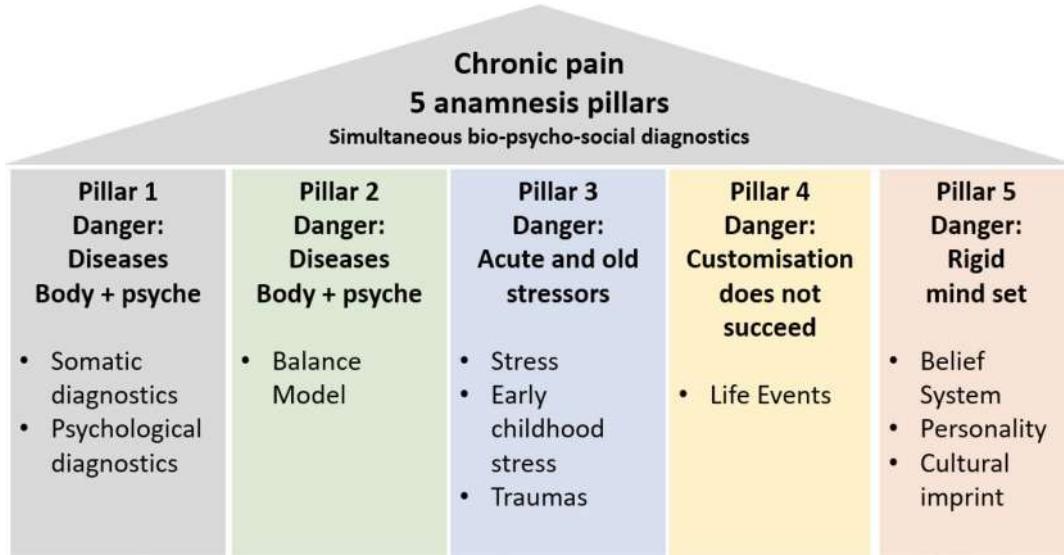


Fig. 18.1 Overview of bio-psycho-social simultaneous diagnostics for chronic pain

Pillar 1—Risk Due to Somatic and/or Mental Illnesses

The first pillar is the need for a detailed physical history, examination and diagnosis. Possible causes of visceral pain must be sought and possibly interdisciplinary collaboration (family doctor, cardiologist, gastroenterologist, urologist, gynaecologist, etc.) must be sought [1]. This process may take some time. It will be helpful to explain to the patient that a somatic search for inflammation, haemorrhage, masses, autoimmune processes, etc. will be carried out in order to be able to initiate targeted therapy. This may also involve surgery and/or medication. At the same time, the other three pillars are investigated and the results collated. According to the work of W. Häuser [7], psychological comorbidities have been identified as confirmed factors influencing the development and course of chronic inflammatory bowel disease (IBD), irritable bowel syndrome and chronic lower abdominal pain in women, so that anxiety and depression must be investigated. Figure 18.2 summarises the diagnosis of the first pillar.

> In the patient example, all somatic diagnostics were performed. Apart from mild gastritis, there were no other pathological findings. The patient did not suffer from anxiety or depression but was severely exhausted.

Pillar 2—Danger Due to an Unbalanced Lifestyle

With the clearly structured balance model according to Nossrat Peseschkian, the four main areas of life (physical health, work, social contacts, values and meaning) can be analysed in a short space of time. It provides a quick overview of the current reality of a person's life: how is their physical health, does the patient work, what social contacts can they fall back on and what sense of meaning do they find in life's challenges? (Fig. 18.3).

The patient can see for themselves which areas are working well and which are not. They can come up with their own ideas to bring their areas of life back into balance. What would you like to do so that your areas of life function better again in 3- or 6-months' time can be an activating question [14].

With the intensified catalogue of questions on the four areas of life, the patient can deal with their own life balance at home, possibly also involving their partner. It has proven useful to give the balance model and the catalogue of questions to the patient to take home. On the one hand, this saves time in the medical consultation and serves to activate the patient's own initiative.

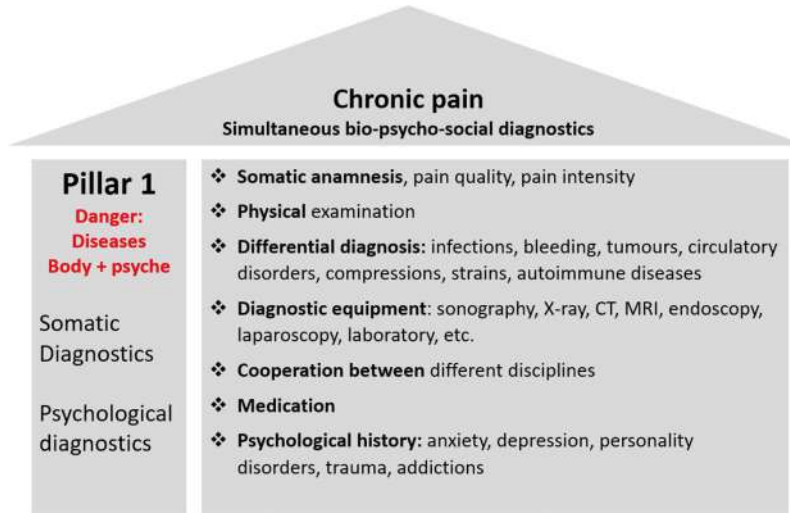


Fig. 18.2 Pillar 1 Diagnostics of physical and mental illnesses

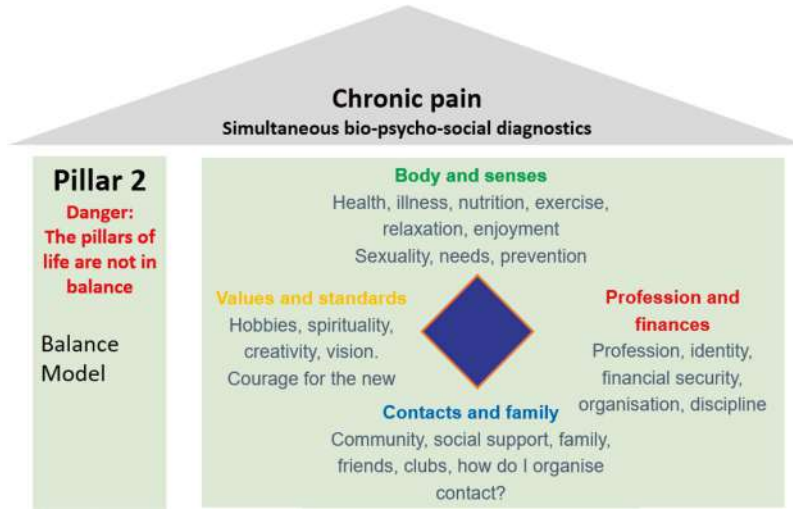


Fig. 18.3 Balance model according to Nossrat Peseschkian [24]

Questions about the four areas of life	
Body and health area What physical illnesses have I had/have? What do I do for my health? Nutrition, exercise, relaxation, prevention, stress management What is my own concept of illness/health? Can I enjoy it? Do I feel comfortable in my body—even as a man or woman?	*Labour and performance* What have I learnt? What are my training courses, qualifications and certificates? What challenges have I already overcome? What experiences have I had? What skills have I developed? Do I maintain order and discipline? Am I financially secure?
Contacts and family Who do I mean when I talk about the "we"? Who can help me? Who can I rely on? Do I have contact with different people? Do I avoid contacts? Am I active in clubs/groups?	*Values and standards* What do I particularly enjoy? Hobbies? Art, music, theatre, literature? What is my goal this year? What traces do I want to leave in this world? Which three wishes would I like to fulfil? Do I live spirituality?

In the patient example, the balance model shows physical exhaustion. The patient is currently on parental leave and is not working and therefore feels financially dependent on her husband. Although she is married, she cannot rely on her husband at all. There are few supportive people around her. She has a positive attitude towards life and is willing to tackle things, so that a change in her life can be achieved quickly.

Pillar 3—Danger from Acute and Old Stressors and Traumas

There are many publications on stress, and it is now common knowledge that massive stress can have a severe impact on mental and physical health even decades later. On the one hand, stress hormones such as adrenaline, noradrenaline and cortisol are released with effects such as increased blood pressure, diabetogenic metabolism, reduced immune defences, minimised fertility, etc. "Stressful experiences can, on the other hand, cause a persistent susceptibility to mental illness", explain Sundari Chetty and Colleagues from the University of California in Berkeley [2, 6]. The brain anatomy can be used to measure permanent changes caused by stress, such as a decrease in the grey matter of the brain. In the prefrontal cortex and hippocampus in particular, the neuronal networks decrease, so that on the one hand the ability to react thoughtfully and appropriately to situations decreases and on the other hand memory function is impaired. On the other hand, the neuronal network in the amygdala area increases, so that attention is increasingly focussed on threatening situations and negative emotions increase.

Early childhood experiences such as domestic violence, addiction, neglect, insecure attachment or sexual abuse also have an impact on brain development and can, for example, lead to somatoform complaints such as sleep disorders,

pain disorders, autonomic dysfunction of the internal organs, dysfunction of the musculoskeletal system and functional complaints. Consideration of early childhood stress is therefore also an important part of the medical history [3].

According to the study, traumatic experiences change the neuronal networks due to the high emotional involvement of powerlessness, helplessness and existential threat. Pronounced vigilance becomes a daily companion with possible numerous trauma-related disorders [4].

However, stress is not only negative, but also leads to creative processes, a high level of motivation and personal growth. Challenges are needed in order to leave one's comfort zone and broaden one's horizon of experience. The increase in personal skills and the feeling of "having done it yourself" in the sense of self-efficacy is then again reflected in the neural network. Figure 18.4.

Figure 18.3 summarises the most important stressors and questions about early childhood stress. These can also be given to the patient to answer at home and encourage further reflection: How do you deal with stress? What would help you to minimise the stressors?

The patient example showed massive stressors such as constant excessive demands, time pressure, relationship conflicts for years, financial dependence on the husband, negative atmosphere in the parental home with parentification (taking on parental tasks) of the patient. However, the patient was also able to study education and had acquired skills by taking on many tasks.

Pillar 4—Danger Due to Lack of Adaptation to Life Events

Thure von Uexküll [19] already called for a "biographical understanding of human illness". Our lives and our health are influenced by life events—both positive and challenging ones. The brain generally links significant events with an emotional soundtrack that can be recalled later. The "tone", whether joy, sadness, anger, offence or hatred, plays an important role in the development of a disease. At the beginning of the 1960s, researchers Holmes and Rahe developed the Social Readjustment Rating Scale, in which they ranked 45 incisive incidents in order of their severity and the necessary adjustment effort. The death of a close relative was associated with 100 LCU ("life changing units")—i.e. with the maximum stress load [9].

[The table shown in Fig. 18.5 enables a self-assessment of the most frequent life events according to the subjective evaluation. This serves the purpose of self-reflection as to whether the life events have already been integrated or

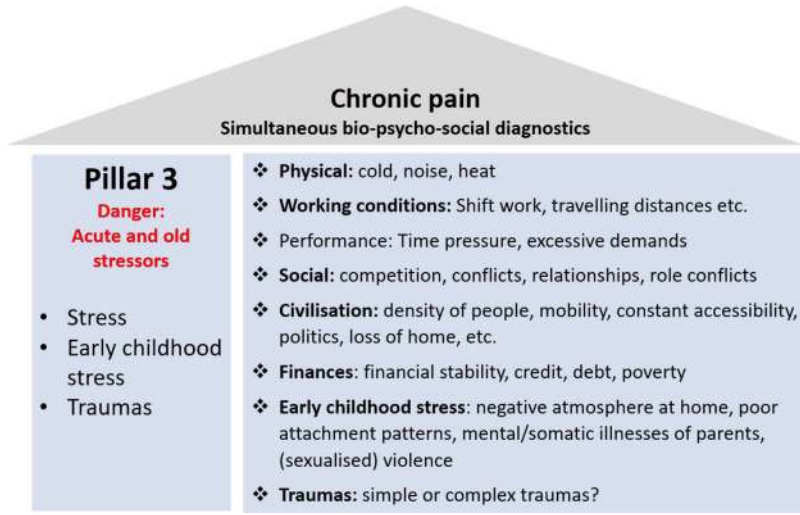
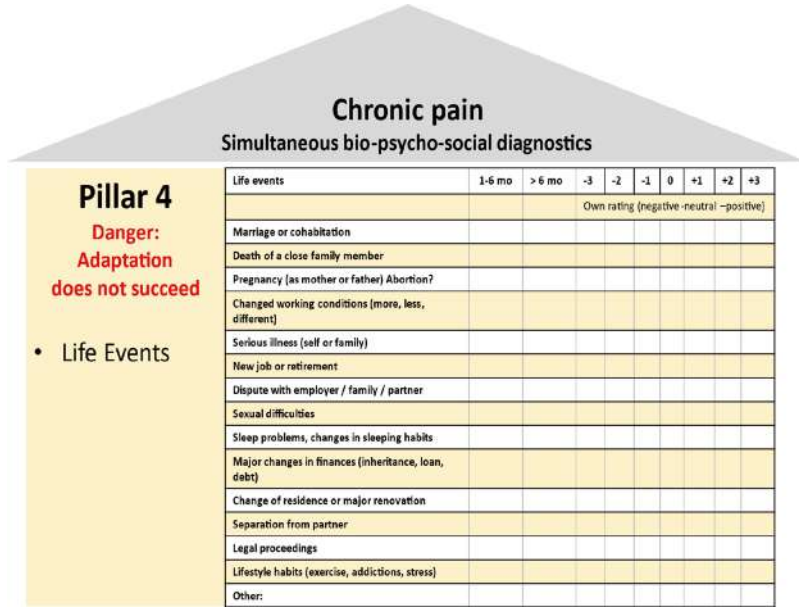


Fig. 18.4 Pillar 3 Stressors and traumas



Life events	1-6 mo	> 6 mo	-3	-2	-1	0	+1	+2	+3
			Own rating (negative -neutral –positive)						
Marriage or cohabitation									
Death of a close family member									
Pregnancy (as mother or father) Abortion?									
Changed working conditions (more, less, different)									
Serious illness (self or family)									
New job or retirement									
Dispute with employer / family / partner									
Sexual difficulties									
Sleep problems, changes in sleeping habits									
Major changes in finances (inheritance, loan, debt)									
Change of residence or major renovation									
Separation from partner									
Legal proceedings									
Lifestyle habits (exercise, addictions, stress)									
Other:									

Fig. 18.5 Summary of the most important life events and the subjective assessment

whether some are linked to negative associations.

The patient experiences the partnership as very stressful with persistent thoughts of separation. She felt left alone and cried at the memory of her two births, saying that no one stood by her side ("my husband went to sleep in the meantime"). She feels financially dependent on her husband but can see her own strength and her own profession as possible independence.

Pillar 5—Danger from a Rigid Mindset

The strength of the influence of our own thoughts and experiences, our own "mind set"in the judgement of a situation, becomes clear in the sentence by Lisa Feldmann Barrett "Our brain is so good at believing in its own inventions that we take the social reality we construct for nature itself" [5]. The more rigidly patients persist in their inner attitude of "I must, I should, I can't", e.g. "I can never work with pain again", the more protracted the support will be, or we as practitioners may have to say goodbye to a curative idea.

Viktor von Weizsäcker described life as "a dynamic, restless becoming with the qualities of experience such as hope, anger, fear, shame, love, annoyance, joy, guilt and grievance" [5, 21, 22] and derived his "pathic pentagram" from this: Wanting, should, can, must, may incl. Their negations not-wanting, not-can, etc. [18]. The more rigidly patients are stuck in the categories, such as "the doctor must take away my pain immediately", or remain stuck in ambivalence "I want to get well, but then I must also take responsibility", the less the pain will be able to change.

Another aspect was identified by Kupper & Dennollet in 1995 under the term Type-D-personality with negative affects and social withdrawal. A negative attitude of "the glass is half empty", strong judgement and competition, as well as passive behaviour increase cardiovascular risk and thus the autonomic nervous system [10] (Fig. 18.6).

In the patient example, the patient was able to work out that she always had to function and be there for others, as a kind of compulsion and guilt. ("If I don't give my all, others will feel bad".) She accepted this as "normal" and did not question it. However, she showed a very flexible mindset and an active attitude, so that it became clear to her that she could also separate from her husband and bring up the children on her own. With this new perspective, the abdominal complaints disappeared.

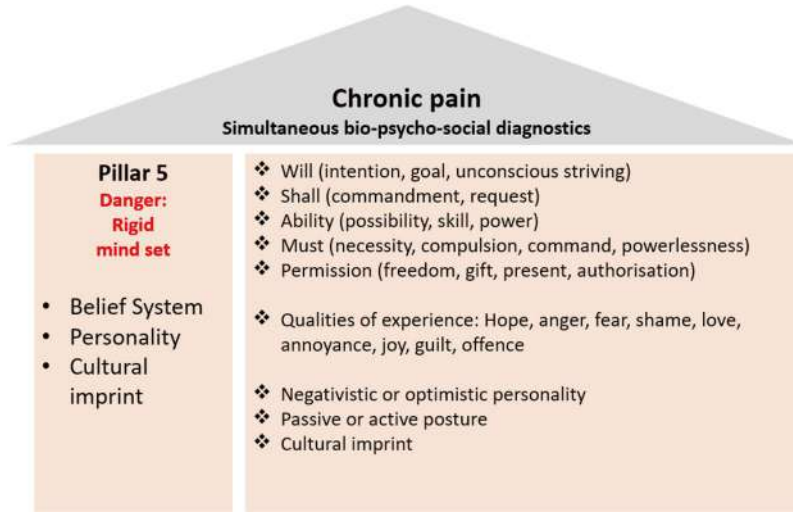


Fig. 18.6 Summary of personality traits and attitudes that can influence chronic pain syndrome

Clear Setting and Treatment Planning

A clear setting and joint (doctor and patient) therapy planning is the key to small-step success. It is important to correct excessive demands immediately and to insist on joint action. Making several proactive appointments shows that there is a genuine interest in the patient and that symptoms do not have to worsen in order to be allowed to go back to the doctor. The following recommendations are made regarding the doctor's attitude and the setting:

Calm attitude	Setting
Objective but emphatic atmosphere	Clear time limit: announce the time available in advance ("we have 10 valuable minutes today")
Questioning and biographical anamnesis serves to improve understanding, not to provide an immediate solution	Do not overload the appointment ("today is about the following point")
Do not give in to the impulse to act quickly (do not react) No redundant medical examinations	Proactively arrange further appointments ("we will meet again in 4 weeks, please complete the biography form by then. I would like to get to know you better")
Don't take on the patient's pressure, correct expectations ("we can't take away the pain completely, but small steps are possible")	Clarification of a joint procedure ("I can do this as a doctor, you can do this as a patient")
Structure the conversation clearly ("We have 3 points today", "I'll summarise again...")	Do not use the word "psycho" for the time being, otherwise, the patient will feel that they are not being taken seriously

Multidimensional Therapy Concept for Patients with Chronic Pain

It becomes clear that patients with somatoform pain are not symptom-free with just one medication. The one-dimensional linear somatic understanding of the disease does not lead to sustainable treatment results. It requires structured, team-orientated, emphatic patient management with realistic small-step goals. It is important to encourage the patient to play an active role in shaping the treatment and to slowly reduce a caring consumer attitude. The focus is also on reflecting successful parameters in life, such as in the case of patient B. "You have achieved great things. You can be proud of yourself". In this

way, the fixation on the negative (including the pain) can be gradually dissolved and other positive perspectives can be included. *Self-efficacy, positive experiences and the motivation to relax, reduce stress, exercise, eat well, expand social skills, have a more positive inner attitude and a sense of purpose are the better medicines*. These points can also become the content of psychotherapy (more focus on relaxation techniques, positive change of perspective, appreciation). Only together can a longer-lasting course of treatment lead to smaller successes.

Summary

In the general population, functional physical complaints are common at 10%, in General practitioners' practices between 25% and 66% patients complain about pain with varying degrees of severity. Reflective dialogue, simultaneous bio-psycho-social diagnostics, good patient information, interdisciplinary cooperation and coping-oriented therapy (activation, self-help, mindfulness-based stress reduction, relaxation techniques, physical activity, etc.) are required standards [1]. The patient's early confrontation with their stress factors, life events and inner attitude can have a positive influence on the recovery process. The structured approach with the five pillars enables a clear, comprehensible procedure and immediately involves the patient as an active participant.

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19

Beyond the Prescription: The Therapeutic Potential of Positive Communication in Healthcare

Ivan Kirillov

Introduction

Alongside medications and surgeries, doctors have long recognized the power of personality and communication styles to heal patients. Stories of great physicians throughout history using effective verbal (metaphors, humor, symbols, stories, provocation, direct and indirect suggestions, etc.) and nonverbal (mirroring, containing, facial expressions, gestures, physical interactions, etc.) interventions continue to inspire both doctors and their patients.

However, modern medicine has largely ignored the influence of social factors on health [1]. The development of a doctor-patient relationship can be traced along the matrix of social evolution into four frames [2].

1.0 In a *traditional, authority-based* frame, the doctor, as the master of medical art and science, relies on experience and established knowledge to diagnose and treat patients. Patients are viewed as subjects to be diagnosed and treated based on the doctor's expertise.

Communication is authoritative, emphasizing the doctor's knowledge and control, leaving the patient in the passive role of a subject to be categorized and "fixed" according to the physician's knowledge: "Doctor knows the best."

This approach offers patients a sense of security and allows for swift decision-making in critical situations. However, it may overlook patients' individuality and resources, potentially leading to feelings of disempowerment, passivity, and disengagement in the process of treatment.

2.0 The *achieving, evidence-based* frame prioritizes statistical evidence and proven so far methods for diagnosis and treatment. Doctors rely on the latest research and technology, positioning themselves as experts in applying established guidelines.

Communication aims to categorize symptoms according to up-to-date statistics and admit the "statistically most effective treatment," minimizing personal involvement and responsibility: "UpToDate knows best!" The doctor's interest in the patient is filtered by statistical criteria.

While this approach offers scientifically sound and unbiased care, it can depersonalize the patient experience, leading to feelings of being just another statistic and demotivation. Additionally, the focus on protocols may overshadow individual nuances and the underlying causes of the illness.

3.0 The *pluralistic, patient-needs-based* frame focuses on collaboration and shared decision-

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_19

making. The doctor, acting as an empathetic partner, considers the patient's needs and actively engages them in the diagnostic and treatment process.

Communication is open and aimed at mutual understanding. The doctor engages the patient in creating a personalized healthcare plan that aligns with the patient's values, capabilities, and resources: "Let's work together to meet your needs."

This approach fosters trust, empowerment, and active patient engagement in collaboration, leading to improved treatment adherence and better health outcomes. However, such a pluralistic collaboration can be time-consuming, and patients who are less accustomed to self-reflection or decision-making may find it challenging to navigate.

4.0 The *evolutionary, self-regulating-system-based* frame focuses on the dynamic interplay of mind, body, social, and behavioral factors in health and illness. Doctors act as facilitators, utilizing their expertise (1.0), up-to-date knowledge (2.0), and understanding of the patient's unique context (3.0) to support the patient's innate capacities for self-regulation.

Communication is focused on encouraging and engaging the patient to explore and enhance their psycho-somato-socio-behavioral resilience, addressing the root causes of disease rather than merely suppressing symptoms "Let explore and utilize your resources of health."

This approach empowers patients to become active participants in their healing journey, fostering long-term well-being. However, it requires active patient engagement, which may not be feasible for all individuals, and necessitates a shift in healthcare delivery toward prevention and health promotion. Moreover, it demands a deeper understanding of mind-body interactions and honed communication skills from healthcare providers.

Recent research supports this tendency to active and professional use of the therapeutic relationship to facilitate the course of illness, activate patients' self-regulation resources, and optimize the impact of medical interventions. Patients themselves mention the importance of a doctor's communication skills more than twice as often as their medical competence [3].

The contextual effects [4] of medical communication induce genuine, reliably predictable psychobiological changes in both laboratory and clinical settings [5]. Properly managed communication can induce the release of endogenous opioids [6, 7], activate the dopamine and serotonin systems [8, 9], emotion-related brain areas [10] and some specific neural pathways [11], reverse DNA methylation marks [12], reduce allostatic load and normalize the microbiome functionality of the gut-brain axis [13].

Especially in the case of pain, functional, motoric [14], and depressive disorders [15], contextual effects may increase the efficacy of admitted medications [16], simulate it (placebo), or even provide clinically significant improvement not associated with any medical or placebo agent [17]. The recent allows us to avoid the ethical complications [18] of placebo usage while directly maximizing the benefits of doctor-patient communication [19, 16].

Unfortunately, despite the above data and the strong correlation of poor physician communication skills with unproductive patient behavior and reduced medicine efficacy down to the placebo effect [20, 21, 22, 23], doctors are still not adequately or only formally and ineffectively trained in these skills [24, 25].

Nevertheless, the growing self-regulating-systemic (4.0) trend in medicine is becoming increasingly recognized, and training health professionals in social and emotional skills has recently deservedly become one of the priorities in health policy initiatives [26].

This progressive intention raises the following questions:

1. How to leverage insights from 4 communication frameworks?
2. Which contextual factors are proven to contribute to treatment outcomes?
3. How does positive psychotherapy meet the challenges of medical communication?

How to Leverage Insights of the Four Communication Frameworks?

It is crucial to recognize that four evolutionary frameworks [2] were sequentially developed. Each builds upon the achievements of the previous one and has its own benefits and limitations. Close observation reveals that these specific characteristics can best address the unique needs of patients according to the stage (see Chap. 2) of their interaction with the health systems and clinicians and the level of their personality integration.

- *Authority-based (1.0) framework:* Ideal for *dependent* patients with low personality integration or those whose functioning has regressed due to stress or illness. These patients need to rely on the support of a trusted authority figure to guide them.
- *Evidence-based (2.0) framework:* Best for patients with moderate personality integration who are striving for autonomy and freedom from limitations (*differentiation stage*). These patients seek objective information and impersonal procedures to manage their health and maintain control.
- *Pluralistic, patient-needs-based (3.0) framework:* Optimal for patients with moderate-to-well-developed personalities and the capacity for introspection and processing complex information. These patients require empathetic understanding, clarification of their needs, and a collaborative, trusting partnership with their healthcare providers as they explore their bodies and develop ownership of their health.
- *Self-regulating-system-based (4.0) framework:* Ideal for patients with well-integrated personalities who are already proactively managing their health. These individuals seek a knowledgeable partner to collaborate with on their journey of self-regulation and optimization.

Understanding the dynamics of these sequential frameworks allows health professionals to tailor their approach to the needs of the individual patient and to foster the development of the patient's personality and relationship with their body conditioning the efficiency of disorders treatment and health promotion.

Which Contextual Factors Contribute to Treatment Outcomes?

Randomized controlled trials on the placebo effect [27] highlighted three key contextual variables significantly impacting treatment outcomes. Each of those can be further elaborated upon, drawing on evidence-based nonspecific factors contributing to the effectiveness of medical and psychotherapeutic interventions.

1. *The Personalities and Beliefs of the Patients* and their support networks [28], including:
   - Ego strength and personality integration.
   - Motivation for treatment and overall health.
   - Capacity to engage in productive relationships with healthcare providers.
   - Disposition for inner conflicts.
   - Capacity for self-awareness and introspection.
   - Ability to identify and address core issues.
   - Beliefs and attitudes toward health and treatment.
   - Level of understanding, support, and encouragement available to the patient.
2. *The Doctor-Patient Relationship* can be even more influential than specific treatment methods. It encompasses several neurobiologically grounded [20, 29, 30, 31] aspects derived from analyzing 16 clinical and meta-analytic studies on nonspecific treatment factors and patient expectations:
   - *Empathy* [32]: Demonstrated through the doctor's availability [33], genuine interest [34], attentive eye contact [33], understanding mirroring and naming of emotions [35, 36], compassionate validation [37], normalization of emotional experi-

ences with a calm reassuring smile [36], and positive interpretation of symptoms' adaptive functions [38].

- *Therapeutic Alliance:* Cultivated by providing a safe, uninterrupted [34] space for patients to express themselves [39, 40, 37], demonstrating understanding and interpreting symptoms convincingly [41, 42], fostering *trust* [32] and patient autonomy, involving patients in decision-making [34], and utilizing their resources [37], encouraging their abilities, and attributing progress to their efforts [41, 43] and self-regulation.
- Patient *Expectation Management* is based on clear, realistic information about the condition and treatment [34], providing *realistic optimism* about outcomes [33] and addressing the patient's actual needs [44, 45]. This involves shifting focus from avoiding symptoms to cultivating health [46].
- Clarity *of Communication:* Asking questions and speaking in a way that is clear and accessible to the patient, considering their vocabulary, understanding, expectations, and needs [37].
- Direct *Care:* Providing specific, necessary help and advice, effective treatment, and referrals when needed [34], based on the patient's needs. Collaboratively creating an action plan [37].

3. Therapeutic Rituals (setting, route of medication administration, taste, name, price, color, procedures regime, etc.) believed to be effective and actively practiced by both the patient and the therapist [37].

How Does Positive Psychotherapy Meet the Challenges of Medical Communication?

Based on feedback from 94 doctors and nurses of different specializations who completed a 15-day training program over 6–8 months, supplemented by 30 h of group meetings for self-discovery and supervision, preliminary suggestions can be drawn about the effective elements that Positive psychotherapy (PPT after Peseschkian, since 1977) can offer to healthcare professionals.

Psychological Orientation Training

Positive psychotherapy can offer a structured, up-to-date, and easy-to-understand theoretical framework to equip healthcare professionals with:

- Enhanced psychosomatic awareness:
  - A clear and informed *understanding of the somatization process* enables clinicians to recognize symptoms' self-regulatory function, better understand patients' experiences, manage their expectations, and engage them in the treatment process.
  - *Evaluation Criteria for Psychosomatic Practice* [47] allow one to differentiate the *main psychosomatic states*[1] and psychosocial factors (*structure vulnerabilities,*[2] *triggers of stress,*[3] and *secondary risk factors*[4]*) that influence the pathogenesis and course of somatic states and illness*es.
- Clear criteria to diagnose the patient's motivation priorities and personality traits (balance model, primary needs, primary and secondary actual capacities) and their specifics at 3 stages of interaction, allowing to adjust the commu-

[1] Conversion symptoms, Associated symptoms, Allostatic overload symptoms, Bodily distress disorders, Secondary somatic symptoms.

[2] Alexithymia, Type A behavior, Key conflict, Misinterpretation of body experiences .

[3] Anniversary/trigger, Stress load of macro- & micro- traumas, Hidden message, Inner conflict.

[4] *Psychological or behavioral factors affecting disorders or diseases classified elsewhere* (ICD-11: 6E40; ICD-10: F-54; DSM-5: 316) are the psychological and behavioral reactions that affect the course and treatment of physical illness, such as Illness denial and avoiding care, Fixation on symptoms, demanding extra care, Factitious disorder, and Demoralization. *In terms of Positive Psychotherapy, one can see this disorder as the capacity to deal with the anxiety related to illness.*

nication style for better engagement, understanding, and adherence.

Competences Training

To apply psychosocial knowledge effectively, healthcare professionals need to develop interpersonal communication skills proven to improve treatment outcomes. Positive psychotherapy, backed by over 50 years of psychodynamic, clinical, and training experience, offers comprehensive training developing the following competencies:

Positive Attitude

The word "*positive,*" understood as "placed"[5] (from the Latin "posit[um]") in the sense of given and real, suggests perceiving symptoms as unconscious attempts of physiological, mental, and social self-regulation [67].

Practically, it implies psychodynamic positive Pavlovian [48] conditioning[6] of healthy constructive responses to a stimulus (triggers, physical symptoms, and experiences) that were previously associated with destructive reactions such as fixation on symptoms, destructive autogenic training, sensorimotor simulation [49, 50], anxiety, fear, worst expectations, demoralization, etc.

Positive suggestions are implemented through:

- *Emotional Presence and Availability:* Offering eye contact, empathically mirroring, and naming the patient's emotions and states translates into full presence, engagement, and understanding, fostering contact with one's own emotions and mentalization.
- *Genuine Interest and Attention:* Actively striving to understand the patient's experiences.
- *Compassionate Validation:* Appreciation of unfolding patient's experiences with calmness, reassuring containing smile (to reduce stress response and promote self-regulation [51]), and verbal confirmation of understanding.
- *Navigating the Patient's Curiosity with Questions:* Encourage the patient to explore their experiences, concepts, dynamics, expectations, and functions of their symptoms.
- *Offering Convincing Positive Interpretation:* Reframing symptoms as a manifestation of the patient's capacity for psycho-somato-socio-behavioral self-regulation.

Salutogenic Focus

Medical doctors are traditionally trained to focus on diagnosing and treating disease symptoms. This approach from the initial patient encounter needs to shift toward emphasizing the identification and cultivation of health markers.

Redirecting patients' attention toward developing and measuring health indicators, rather than solely focusing on avoiding symptoms of illness, leads to significant improvements in physical function, treatment outcomes [46], adherence to treatment plans, quality of life, sense of coherence [52], resilience [53], and coping strategies [54]. These positive effects are particularly noticeable in mental health and chronic disease management [55].

To navigate the focus shift toward health, positive psychotherapy offers a balanced model for systematic exploration of the patient's concept of health and management of positive, realistic expectations of clinical improvement that proved to activate the mesolimbic dopaminergic system [56], reduce anxiety, boost faster recovery [33]. This model helps specify indicators of physical, social, mental, and behavioral health and to identify appropriate practices to improve each of these aspects.

[5]Positive /ˈpɒzɪtɪv/: consisting in or characterized by the presence rather than the absence of distinguishing features. (https://en.oxforddictionaries.com/definition/positive). Positum (lat.)—placed, positioned (ABBYY Lingvo, Oxford Latin Dictionary, 1982; Latin Dictionary by Enrico Olivetti).

[6]Pavlovian conditioning involves associating a neutral stimulus (like a placebo pill) with a desired biological outcome (like pain relief). Eventually, the neutral stimulus triggers the biological response, even without the original cause.

Trust and Encouragement

Trust and encouragement have been proven to be powerful competencies for engaging the patient in the treatment process and boosting the best outcomes of specific medical procedures and interventions. One can boost this competence by practicing:

- *Attributing Success to the Patient's Efforts, Capacities, and Resources* [41].
- *Encouraging the Patient's Coping Capabilities* and helping optimize them.
- *Trust the Patient's Needs (Motivations), Capacities, Perceptions, and Inner Knowledge* to develop their self-help tools.
- *Engage the Patient in Informed Decision-making* about the strategies of treatment.
- *Make Clear Agreements and Fulfill Promises*.

Mature Communication

Positive psychotherapy offers some useful frameworks and strategies for mastering interpersonal communication.

- *Creating a Safe Space and Time for the Patient* to unfold and explore their experiences through their unique processes. Release any expectations.
- *Tailoring Language and Communication Style* to the patient's culture, values, understanding, personality traits (accentuations) and integration, actual state, and stage of interaction.
- *Mastering Usage of Symbols, Metaphors, and Humor* as media to patient's inner narrative.
- *Navigating the 3 Stages of Interaction and the 5-Stages Strategy* to model and optimize the patient's inner algorithms of experiences.

Ongoing Supervision

Communication skills ongoing training for physicians, enhanced by role-play, feedback, and group discussions, improves doctor-patient interactions and treatment outcomes [25] and reduces physician burnout. However, to maintain and develop interpersonal skills, clinicians need ongoing self-assessment and supervision of their regular practice of clinical communication competencies [57]. Here positive psychotherapy can offer a well-structured and proven effective [58] supervision model (see Chap. 37).

Mental Hygiene

Personal experiences and self-perceptions of medical doctors have been shown to influence treatment outcomes [59]. Research indicates that physicians' mental well-being, enhanced through Balint groups [60], self-discovery, dialogue [61] and optimization groups, or individual psychotherapy [62], positively influences their efficacy, patient care [63], and medical decision-making [64]. Positive Psychotherapy offers structured self-discovery [65, 66], group (Chap. 25), and individual therapy (Chap. 2) frameworks to support physicians in this regard.

Summary

The chapter explores the evolving landscape of doctor-patient communication, tracing its progression from traditional, authority-based models to a more contemporary, evidence-based, patient-centered, and self-regulation-focused approach. It highlights the significant influence of effective communication on treatment outcomes, emphasizing that this extends beyond conventional medical training and expertise.

The chapter delves into the critical role of contextual factors, such as patient personality, the quality of the doctor-patient relationship, the impact of therapeutic rituals, and salutogenic focus in shaping treatment success. It underscores the potential of Positive Psychotherapy in equipping healthcare professionals with the necessary communication skills and psychosomatic understanding to navigate these complexities.

Furthermore, the chapter stresses the importance of ongoing training, supervision, and mental hygiene for healthcare providers to continually refine their communication skills and ultimately optimize patient care and treatment outcomes. It

advocates for a shift in focus from solely treating disease symptoms toward salutogenic health promotion, fostering patient self-regulation, and acknowledging the dynamic interplay of mind, body, social, and behavioral factors in health and illness.

Key Points

- *Evolution of Doctor-Patient Communication* from a traditional, authoritative model to an evidence-based, patient-centered, and self-regulation-focused approach.
- *The Impact of Communication on Treatment* is crucial for positive treatment outcomes, going beyond conventional medical training and expertise.
- *Contextual Factors* such as patient personality, the doctor-patient relationship, therapeutic rituals, and salutogenic focus correlate with significantly improved treatment success.
- *Positive Psychotherapy* offers a valuable toolkit for enhancing healthcare professionals' communication skills and psychosomatic awareness.
- *Ongoing Training, Supervision, and Mental Hygiene* are essential for healthcare providers to maintain and improve their communication skills and the outcome of the treatment they provide to their patients.
- *Salutogenic Shift in Focus* fosters patient self-regulation and health promotion, considering the complex interplay of mind, body, social, and behavioral factors helps manage realistic positive expectations from the treatment, improving its efficacy.
- In essence, the chapter highlights the pivotal role of effective communication in healthcare and advocates for a more holistic, patient-centered approach that empowers patients to actively participate in their healing journey.

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Part IV

Special Settings and Populations

Minorities and Culture: Positive Psychiatry and Psychology Perspectives

20

Gina Newsome Duncan, Kirsy Japa, Shalice McKnight, and Rama Rao Gogineni

My mission in life is not merely to survive, but to thrive; and to do so with some passion, some compassion, some humor and some style.

(Maya Angelou).

Introduction

Positive psychology is the study of positive emotion, positive character traits, and enabling institutions [12]. Over the past two decades, the field of positive psychology has grown substantially and shown great promise as a refreshing approach to the field of mental health care as well as fields such as education and business [1]. Positive psychiatry is closely allied with positive psychology and shares many overlapping constructs and aims [2]. Although variations between positive psychiatry and psychology can be seen due to the diverse methods of interventions employed by practitioners based on their education, and the wide range of individuals receiving these interventions [1, 53]. New approaches to mental health care are greatly needed, as our society is increasingly faced with mental health crises that a pathology-focused approach has not been able to sufficiently address [3, 4].

As mental health clinicians, we should aspire to provide every individual we serve with skills, tools, and corrective emotional experiences that enable them to achieve optimal human functioning. Positive psychology, psychotherapy, and psychiatry add to our traditional mental health armamentarium by providing an understanding of the factors that enable people to flourish and providing tools to assess and build these individual strengths [2, 3, 12]. This chapter focuses on positive psychology and psychiatry. For positive psychotherapy approaches across different cultures, see Chap. 21.

Positive psychiatry as a science and practice aims to understand and promote well-being in individuals experiencing, or at risk of developing, mental or physical illness through assess-

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_20

ments and interventions centered around positive psychosocial characteristics (PPCs) [2]. As outlined by Dilip Jeste and collaborators, positive psychosocial characteristics (PPCs) include traits such as resilience, optimism, social engagement, and spirituality, all of which have been associated with better health outcomes and subjective well-being [2]. As a branch of medicine, positive psychiatry also seeks to better understand the biological bases of such traits and to develop preventive psychosocial and biological mental health interventions [2]. Positive Psychotherapy described by Nossrat Peseschkian (1933–2010) is a resource-oriented psychotherapeutic treatment method for individuals, couples, and families is a humanistic and transcultural psychodynamic psychotherapy and is based on a positive conception of human nature and salutogenetic principles [17].

In positive psychology theory, well-being, the experience of flow, and positive emotions such as happiness rest on foundational virtues and character strengths [4, 13]. The theory is also rooted in how individuals perceive and interpret their subjective experience across time—past, present, and future to enhance life satisfaction and fulfilment [2]. Six virtues are considered to be universal to all cultures around the world: wisdom, courage, humanity, justice, temperance, and transcendence [4, 13]. Each virtue has several associated character strengths, making 24 character strengths in total [4, 13]. According to Peterson and Seligman, each individual possesses all 24 character strengths but to varying degrees [13]. When individuals understand and operate from their character strengths, they experience subjective well-being [13].

Martin Seligman's PERMA theory describes five elements that contribute to well-being: positive emotion, engagement, relationships, meaning and purpose, and accomplishment [14]. Character strengths are the backbone of the PERMA theory [15]. When an individual knows and acts on their strengths, it leads to more positive emotion, more engagement, better relationships, a greater sense of meaning and purpose, and the opportunity to experience accomplishment [14, 15]. When all components of the PERMA model are being lived to the fullest, the result is *flourishing* [14, 15].

Therapeutic interventions based on positive psychology theory seek to increase positive emotion, engagement, and meaning and have shown benefit in the treatment of depression [16]. Positive psychotherapy (PPT) is a short-term humanistic form of psychodynamic psychotherapy developed by Dr. Nossrat Peseschkian that emphasizes activation of patients' innate therapeutic capacities. See Chap. 2 for a summary of Peseschkian's positive psychotherapy model [17, 18].

Cross-Cultural Applications of Positive Psychology and Psychiatry

Traditional models of care have been ineffective at fully reaching multicultural populations in the United States, and there are significant disparities in mental health care, [5, 6] including ethnic and racial [3, 54]. Contributors to these disparities are barriers at the level of the individual, the community, and the health-care system and include pervasive issues of mistrust, stigma, systemic inequality, and a lack of clinicians of color [7, 19]. In their 2008 article, Sriwattanakomen et al. called for the development of culturally relevant preventive mental health interventions that are both practical and sustainable in order to overcome disparities in mental health-care services [7].

With its emphasis on strengths over deficits, and its core built on virtues shared by most cultures across the world [4], positive psychology can serve as an effective foundation for such interventions if applied in a culturally responsive manner, by acknowledging the cultural differences, and identifying cultural traits, that can be incorporated into the positive psychology and psychiatry approach [3, 58]. Unfortunately, to date there has been a relative paucity of studies exploring positive psychology constructs and interventions within populations of color [8]. But the mental health clinician seeking to provide positive psychology-oriented care in a culturally relevant man-

ner need not lose heart. Studies done to date [20] and explorations of cross-cultural values and norms [8, 9, 21, 22] strongly suggest that positive psychology interventions (PPIs) have the potential to be applied in effective, culturally congruent ways. However, several issues affecting cross-cultural application of positive psychology and psychiatry should first be explored [9].

One issue is to recognize that in order to most effectively apply a positive psychology approach in multicultural contexts, it must be acknowledged that positive psychology itself arises from a specific cultural frame of reference (Western/European-American) [9]. As a result, it may not fully encompass the values and experiences of ethnic minority/non-Western cultures, most of which are fundamentally collectivistic rather than individualistic [9]. Collectivistic cultures are characterized by additional key positive values, strengths, and traits which complement those held by mainstream positive psychology [8, 9]. Collectivistic cultural values/strengths; include social support, being other-focused rather than individually focused, and a central importance of spirituality and religion [8, 9, 23, 24]. An abundance of existing literature supports the significant role these key cultural elements play in supporting the mental health of people in collectivist cultures [8, 9, 23, 25, 26].

A second issue is to recognize that for many people of color in the United States, opportunities to truly flourish may be blocked by factors such as systemic discrimination, racial microaggressions, poverty, and limited educational opportunities [9, 27, 28]. For Latinos, access to resources to take an active role in their mental health journey is limited by inadequate use of mental health services stemming from cultural perceptions, language barriers, socio-economic factors, the rise in the number of immigrants from different backgrounds, and specific policies enacted in the healthcare reform [55].

A third issue is to understand that people of color in the United States often possess what Constantine and Sue describe as key *adaptive* strengths which are the direct result of their experiences as minorities living in and overcoming discrimination and adversity [9, 11].

Also, the need to recognize and focus on racism alone might neglect intra-group diversity. A study in which African American participants were recruited from an online crowdsourcing platform illustrates these issues. A multivariate multiple regression examination of the effects of racism, classism, and the interaction of racism and classism on depression, stress, and well-being endorsed both racism and classism influenced mental health outcomes. Findings suggested that experiences of classism can be detrimental to the mental health and well-being of African Americans. But attention to both racism and classism contributed to better mental health and well-being [56].

Positive psychology and psychiatry interventions that (1) acknowledge the effects of historical and ongoing systemic inequality, (2) explore and emphasize collectivistic values, and (3) recognize key adaptive strengths have the potential to be powerful tools for cross-cultural mental health promotion [9, 11, 29]. Furthermore, helping people tap into and apply their individual and collective strengths with greater intentionality may empower individuals and their communities to further overcome the systemic barriers they have historically faced.

The Necessity of Self-Examination

Successfully implementing a culturally competent positive psychology or positive psychiatry approach in clinical care requires that the clinician not only has a solid understanding of core positive psychology and psychiatry concepts but also engages in self-examination around several key areas. The first area has to do with self-examination around the extent to which one, as a clinician, is truly coming from a strengths perspective and is committed to carrying it out in patient care [8].

Saleebey has written extensively on the strengths perspective and how it compels clinicians to look beyond the pall of adversity to truly see people in the full context of their abilities, ideals, and potential [30]. Positive psychology is inherently a strengths-based

approach [4]. Much of the positive psychology literature to date has focused on understanding and developing personal character strengths in order to help individuals achieve well-being [8, 9, 12, 13, 31–33]. However, literature on the strengths perspective (which comes out of the field of social work) places additional emphasis on understanding, appreciating, and drawing upon the resources available in the broader community of which the individual is a part [30, 34]. This is of central importance in a discussion of positive psychology's applicability to the mental health care of people of color in the United States, particularly those who are from underserved or marginalized communities. For example, a patient from an underserved, marginalized minority community in the United States may not present with the measures of success many of us as clinicians are accustomed to associating with certain character strengths and which we can readily identify in our more affluent and better-educated patients, in our peers, and in ourselves. In some cases, this may be due to the individual not having had the opportunity to fully display character strengths such as *creativity, leadership, or love of learning* in ways that are easily identifiable, directly as a result of systemic barriers such as poverty, a poor quality of education, and discrimination [8, 9, 11].

The second area around which clinicians must examine themselves has to do with their commitment to cultural competence. The model of cultural competence in health-care delivery developed by Josepha Campinha-Bacote consists of five constructs:

1. Cultural awareness.
2. Cultural knowledge.
3. Cultural skill.
4. Cultural encounters.
5. Cultural desire [35].

While each of the five constructs is essential, we would like to highlight the construct of cultural desire in particular because it specifically speaks to the health-care clinician having an authentic passion to learn and grow from an understanding of cultural similarities, differences, and nuances [35]. In culturally competent care, the clinician makes a concerted effort early in the treatment relationship to learn about the patient's cultural orientation and religious/spiritual beliefs [35–38]. This means going beyond knowing the "what" of the individual's culture or beliefs to understanding the "how." In other words, knowing that someone is Chinese American and that they are Buddhist is a "what" level of knowledge. A culturally competent clinician seeks to understand *how* those identities impact the person's perceptions of their strengths, their purpose in life, and what they see as the meaning behind their suffering or challenges. Clinicians cannot assume that they know the "how" even if they are familiar with the surface-level "what" [36, 37].

The third area around which clinicians must examine themselves is their ability to recognize the adaptive strengths possessed by minority groups in the United States and how these adaptive strengths contribute to individual and collective well-being [8, 9, 11]. Despite experiences of significant discrimination and oppression, people of color in the United States have demonstrated considerable ingenuity, drawing from their own unique cultural traditions—emphasizing family, community, and collectivist principles, using spiritual and religious perspectives to make meaning of adversity, and engaging in deliberate practices to instill ethnic identity and pride—to become resilient [19, 29, 39–41].

Resilience is the capacity to thrive in the face of significant adversity [42] and can be viewed as a particularly defining characteristic of the African American experience. While enduring the dehumanizing conditions of bondage over centuries, enslaved African Americans maintained a healthy sense of self-identity that enabled them to push for emancipation and never accept enslavement as a permanent reality. This same characteristic has been a central driving force behind the ongoing struggle for civil rights in the United States. Resilience for African Americans has been inseparable from their innate spirituality, rooted in African cultural tradition,

and sustained and fostered by institutions such as the Black Church [43]. Constantine and Sue have discussed additional psychological adaptive strengths developed by people of color in the United States in order to navigate life in the face of discrimination and oppression [9, 11]. These include heightened perceptual wisdom, ability to rely on non-verbal and contextual meanings, and bicultural flexibility [9].

The fourth area of self-examination requires one to eschew notions of the superiority of one cultural frame of reference or belief system over another.

It is critical that clinicians are aware of the assumptions they bring into treatment with patients [34, 36, 38]. Clinicians must recognize that appraisals of strengths and optimal functioning based solely on one's own personal or cultural frame of reference can be deceptive [38]. As an example, for young adults in American society, moving out after graduating from college and building a career and family in a separate household from one's family of origin are generally expected and a sign of a successful "launch" [11]. If we were to consider young adults we know (whether in our personal or professional lives) who have successfully navigated this transition, it is likely that we could easily associate certain positive psychology character strengths with them (e.g., *zest, love of learning, self-regulation, judgment, and perseverance* to name a few). In my practice (Gina Duncan, MD), a generous subset of patients are young adults in their early to mid-20s who present with their parents with a chief concern that they have had a "failure to launch," because they are still living at home with their parents following unsuccessful attempts at going away to college or moving out on their own. This is a central concern for their parents and a source of depression, anxiety, and low self-esteem for them. By contrast, young adults from other cultural backgrounds, particularly immigrant communities in which interdependence is a central value, may experience the opposite expectation; they may be expected to remain at home and attend to family obligations while pursuing higher education and starting their career [11, 44]. When clinicians are unaware of the impact of their own cultural background, values, beliefs, and privilege, they can unknowingly pathologize the perspectives, behaviors, and beliefs of others [38]. The ADDRESSING model proposed by Dr. Pamela Hays provides an excellent framework for examining the many dimensions of culture, including one's own cultural orientation, areas of bias, and privilege [38].

The final area of self-examination requires the clinician to acknowledge that in every clinical encounter, there are always two experts in the room [45]. The clinician possesses a level of expertise based on their training, but the patient is the expert when it comes to their own history, values, and preferences [45].

Stigma, Culture, and Mental Health

Though anti-stigma campaigns are showing effectiveness in raising awareness around mental health, stigma is an unfortunate but very real barrier to acknowledging mental illness and to seeking mental health treatment in many societies [2, 45]. In Western societies such as the United States, it is important to recognize that stigma can impact the mental health of minorities and persons of color in complex ways.

Constantine and Sue have discussed the circular and interconnected context in which many cultures view human behavior and its consequences [9]. Because many cultures are highly spiritual and view mental, physical, and spiritual health as interrelated, mental health problems may be perceived as having a spiritual origin [8, 9, 23, 26, 40, 43, 46, 47]. For example, symptoms of mania or psychosis may be interpreted as evidence that a malevolent spirit is at work, or the inner turmoil associated with anxiety and depression may be viewed as punishment for sins or as evidence of a lack of faith. As discussed by Pargament and Mahoney, negative religious reframing can lead to negative experiences such as guilt, blame, and denigration [48]. Such experiences can further intensify emotional pain, increase stigma around mental illness, and

decrease individuals' comfort with non-traditional forms of help-seeking [5, 7, 19]. The recovery literature has shown that mental healthcare clinicians also contribute to stigmatizing experiences for persons receiving mental health services [45]. This has largely been related to clinicians' use of stigmatizing language and labels that convey a sense of the person being defined by their illness [45].

Positive Psychology Interventions (PPIs)

Positive psychology interventions (PPIs) are intentional activities designed to increase subjective well-being by inducing positive affect, thoughts, and behaviors and by minimizing negative affect [33, 49]. They may be used in the context of psychotherapy to supplement traditional interventions or as self-help strategies [10]. Frequently discussed PPIs include the *Gratitude visit, Three good things in life, You at your best, Using signature strengths in a new way,* and *Identifying signature strengths*.

Seligman et al.'s 2005 study of positive psychology interventions is a helpful study to highlight [10]. The authors conducted a 6-group, random assignment, placebo-controlled Internet study of 577 adults. They found that individuals who experience the greatest life satisfaction are those who orient their life pursuits toward engagement, meaning, and the experience of positive emotion, with the greatest weight being carried by engagement and meaning [10]. Participants were randomly assigned to one of five happiness exercises, the *Gratitude visit*, *Three good things in life*, *You at your best, Using signature strengths in a new way,* and *Identifying signature strengths*, or one placebo-controlled exercise: *Early memories* [10]. Participants were, on average, mildly depressed based on baseline CES-D (Center for Epidemiological Studies-Depression Scale) scores [10].

In the *Using signature strengths in a new way* exercise, participants were asked to take a strengths inventory at www.authentichappiness.org and to then use one of their "signature" strengths daily for 1 week in novel and unique ways [10]. In the *Three good things in life* exercise, participants were asked to make a nightly practice of writing down three things that went well that day and the reason [10]. In the *Gratitude visit*, participants were asked to write and then hand-deliver a letter expressing thanks to someone who had been particularly kind to them [10].

While the *Gratitude visit* was shown to induce positive changes for 1 month, *Using signature strengths in a new way* and *Three good things in life* were found to increase happiness and decrease depressive symptoms for 6 months [10]. Participants who continued to practice the exercises demonstrated the most significant benefit in terms of improved happiness scores [10].

While the study itself provides very helpful information on how positive psychology interventions (PPIs) might be implemented, a primary limitation of the study is that the sample was predominantly White, well-educated, and financially well-off [10]. This makes direct generalizability to minority, lower-income, and less educated populations difficult [10].

Though not directly focused on cross-cultural applications of positive psychology, Sin and Lyubomirsky's 2009 meta-analysis is also a helpful study to highlight because their findings encourage broad use of PPIs in depressed and non-depressed patients and because they speak to the importance of designing culturally relevant PPIs [20]. In the meta-analysis, the authors examined 49 PPI studies across 4266 individuals [20]. They found that PPIs significantly enhance well-being and are effective for treating depressive symptoms [20]. Therefore, they recommend that PPIs be implemented broadly in the treatment of patients in general, including those who are currently depressed or experiencing residual depressive symptoms, those who have recovered from a depressive episode, and those who are euthymic, as both depressed and non-depressed patients are likely to benefit from these strategies [20].

Furthermore, they provide three helpful suggestions for clinical practice:

1. Patients should be encouraged to maintain regular practice of their PPIs and keep a record of them so as to turn them into habits [20].
2. Practicing multiple different PPIs may be more beneficial than engaging in only one activity [20].
3. It is imperative that clinicians take into account a patient's cultural background when implementing PPIs, as members of collectivist cultures may benefit more from PPIs that are focused on others (e.g., acts of service) rather than focused on self (e.g., reflecting on personal strengths) [20].

Specific Applications and Clinical Examples

In keeping with Seligman et al., we recommend that clinicians think in terms of enhancing patients' experiences on each dimension of the *PERMA* (*Positive emotion, Engagement, Relationships, Meaning and purpose, Accomplishment*) model [10], working to engage the patient in identifying activities that are of greatest relevance to their cultural and spiritual perspective [20]. As an example, for an African American Christian patient, increased church involvement may provide a prime opportunity to enhance the dimensions of PERMA. Participating in corporate worship, prayer, and Bible study groups can enhance connectivity with others and increase social support by strengthening established relationships and fostering the development of new ones [50]. Such activities can also enable individuals to share their struggles and celebrate their successes with others and to see that they are not alone. Engaging in church missions such as a clothing drive, soup kitchen, or homeless ministry may help promote further engagement with the community and a sense of accomplishment. Furthermore, providing acts of service to those who are less fortunate may help to foster feelings of gratitude, as such activities often cause us to reflect on the blessings and resources we have. Participating in corporate prayer, praise, and worship and listening to sermonic messages may also help to reinforce a sense of subjective well-being from a spiritual standpoint by allowing individuals to reflect on the struggles they have had in the past and the ways in which God has provided for them [C.G. Newsome, Ph.D., personal communication]. Such experiences can enable individuals to derive a sense of meaning from their struggles.

Another example we can illustrate "to enhance the dimensions of PERMA", includes specific Latino traditions such as "Quinceanera fiestas and Celebration", an elaborate and ritualized fiesta, that signifies the transition into womanhood [57], that can align with each dimension of the model. Throughout this occasion, engaging with music and traditional dances, along with sharing traditional food can evoke feelings of nostalgia and joy, fostering positive emotions and a sense of involvement. This celebration emphasizes the importance of family connections, relationships, and support networks in the community, which play a crucial role in fostering a sense of belonging and identity in the Quinceanera. Participating in quinceanera traditions highlights cultural pride, family love, personal growth, and instills a sense of significance and direction.

Patients who are not religious may still benefit from exploring and connecting with their own innate sense of spirituality by taking inventory of the activities they engage in that promote a sense of purpose and oneness with nature, with others, and/or with a Higher Power [8]. For some patients in minority communities, civic engagement and working toward issues of social justice may be a central way to enhance connectedness, meaning, and purpose. Such activities may help to offset the challenges associated with being in a marginalized group by building a sense of empowerment.

A Notable Paradox in Native American Communities

Some American Indian/Alaska Native communities exhibit high rates of suicide. Research has shown that hope and optimism are negatively related to sui-

cidal ideation while substance use, psychiatric disorders, and cultural identity are seen as risk factors. Protective factors include spirituality and social support or belongingness. A study to investigate hope and optimism in relation to suicidal ideation, thwarted belongingness, perceived burdensomeness, and acquired capability showed that hope and optimism negatively predicted thwarted belongingness, perceived burdensomeness, and suicidal ideation. These results were not found for acquired capability. The study that included American Indian/Alaska Native participants from three Midwestern universities (women, men, married, single, cohabitating, divorced, separated) from 27 different tribes suggested that higher levels of hope, agency, and pathways are related to lower levels of thwarted belongingness, perceived burdensomeness, and suicidal ideation [58].

Another study demonstrated a notable mental health paradox in two American Indian tribes, a cultural group that have experienced historical trauma, discrimination, and disproportionate levels of chronic diseases, including type 2 diabetes and mental illness. The Mino Giizhigad (A Good Day) study included the Lac Courte Oreilles (LCO) and Bois Forte (BF) Bands of Chippewa. The people in these two communities were shown to have a paradoxically high prevalence of positive mental health. This evidence of disproportionately high positive mental health highlights a source of strength and wellness for people in these communities. Further, findings support the view that discrimination is negatively associated with positive mental health while traditional activity involvement is positively associated, underscoring the importance of conceptualizing and measuring culturally unique sources of stress and resilience across racial/ethnic minority communities [59].

Clinical Example #1

> D.L. is an African American woman in her 50s who has been disabled for the past decade due to mental illness. Her personal struggles have included economic stressors, domestic violence, a chronically ill family member for whom she is a primary caretaker, and exacerbations of her own physical and mental illnesses. Despite this, she is a very active member of her community. She manages her own mental health struggles in part by being deeply involved in the community as a foster parent and neighbourhood watch leader, attending city council meetings, and participating in voter registration efforts.

Engaging in community activities that help to celebrate and educate others on one's culture may also be beneficial. Such activities could include being involved in programs where younger members of the community are educated on their history and heritage. Doing so provides the patient the opportunity to build relationships with others, to "give back" in a meaningful and purposeful way by building the ethnic pride and identity of the next generation, and to reinforce their own sense of ethnic pride [8]. Such activities may help to promote subjective well-being and positive emotions individually as well as collectively.

Clinical Example #2

> C.B. is a middle-aged woman and first-generation American who is the daughter of immigrants from Central America. Her family history is significant for severe mental illness, alcohol dependence, and domestic violence. Despite the patient's own mental illness, and despite financial limitations, she tirelessly serves in her local church helping other immigrant families and participating in local cultural festivals that allow her to showcase her heritage and the strengths of her community. She views her community and church engagement as part of her therapeutic process as well as her Christian duty.

As in Clinical Example #1, engaging in community activities, especially same-ethnic community activities, can promote subjective well-being and positive emotions, and this is sometimes hard for the children of immigrants who may live "in between worlds." Religious institutions can be instrumental in providing a space for this engagement and connection.

Clinical Implementation of Culturally Relevant Positive Psychology Interventions (PPIs)

Step 1 Detailed Psychosocial History Including Cultural History, Religious/Spiritual Beliefs, and Strengths Inventory

Effective treatment rests on the foundation of a solid rapport and trust between the patient and clinician. We recommend beginning treatment with a detailed psychosocial history. Doing so allows you to familiarize yourself with the patient's experience and the ways in which their cultural background, religious/spiritual perspectives, and other experiences have influenced their health beliefs [38]. We recommend taking your time in gathering this history and explaining to the patient why you are taking this approach. For example, you might state: "In order for me to best help you, it is important for me to have a good understanding of who you are as a person and the different experiences and influences that have shaped who you are today."

We recommend that the initial assessment of strengths be approached in a less formal way and be incorporated as part of the initial process of building rapport and gathering history. In our experience, patients who are very depressed or demoralized are likely to have significant difficulty identifying positive traits in themselves. A critical role of the clinician in this scenario is to help the individual move into a healthier frame of mind by reflecting back to them the positive traits we see in them. We cannot do this in an authentic way if we have not gotten to know the patient as a whole person or established an effective rapport. Xie has outlined practical strategies for approaching an initial strengths assessment in a therapeutic encounter [34]. In particular, Xie notes that if the individual is having difficulty identifying their strengths, informal conversation with general questions about their hobbies, activities they enjoy, or how they have gotten through acute phases of their illness in the past can be illuminating [34].

In addition to gathering details of the patient's family history and significant experiences of childhood and early adulthood, we recommend devoting attention specifically to an exploration of their unique cultural background and heritage. This process enables the clinician to gain insight into the patient's cultural norms and the extent to which they identify with collectivistic vs. individualistic values [8, 34, 38]. Invite the patient to be curious with you as you engage in this exploratory process, paying particular attention to personal and collective cultural narratives of overcoming adversity. Doing so may have therapeutic benefit and enhance well-being by providing inspiration and meaning [30]. Lopez et al. have noted that leading a patient through an exploration of their cultural background has been shown to promote the development of self-esteem in Asian-American, African American, and Hispanic adolescents [8]. For many people of color, identifying with the struggles of ancestors who overcame adversity instills a sense of pride and can be another source of strength, encouraging them that they, too, can meet the obstacles they face. For Latinos, cultivating a strong positive cultural and racial identity, along with a close supportive and cohesive family and community system, appears to be essential for fostering individual resilience when confronted with social disadvantages, disparities and injustice [4, 56].

Finally, it is important to explore with the patient their own sense of spirituality and their religious beliefs [8]. This entails going beyond screening-level questions which ask simply whether faith or spirituality are important to the individual or whether they use prayer as a coping strategy. Go further by engaging the patient in a discussion of what they believe the meaning of their struggle is. If approached in a sensitive manner, this can initiate a helpful dialogue leading to deeper understanding for both the patient and the clinician. Questions such as "Why do you think you are going through this?", "What do you believe God is trying to show you through this experience?", and "Could you say more about the ways in which you feel your faith supports you?" are examples. The goal is to get

the patient connecting more deeply to their beliefs and, if they have not previously been spiritually in tune, to open an opportunity for them to explore their intrinsic spirituality.

Key Questions to Explore a Patient's Faith on a Deeper Level

- Why do you think you are going through this experience?
- What do you believe God is trying to show you through this experience?
- Could you say more about the ways in which you feel your faith supports you?

Following the detailed psychosocial history or concurrent with it, the patient can be directed to a resource such as the VIA Survey of Character Strengths and other questionnaires at www.authentichappiness.org [51]. The results of the surveys as well as the information gathered from the psychosocial history can then serve as the foundation for the development of individualized PPIs.

Step 2 Design Culturally Relevant PPIs with the Patient

Once the psychosocial history and survey of strengths is completed, we recommend reviewing with the patient all strengths and observations that have emerged during the assessment. Then, in concert with other necessary therapeutic interventions (e.g., medication and additional psychotherapeutic techniques as appropriate), assist the patient in designing PPIs that are in sync with their unique cultural identity, spiritual frame of reference, and personal goals. Xie notes that this can require a shift for the clinician, who may be accustomed to designing treatment interventions based on what the clinician perceives to be most important [34]. However, this is very much in keeping with the concept of shared decision-making [45].

Cultural celebrations can provide helpful material for the development of unique, culturally relevant PPIs. For example, the African American celebration of Kwanzaa provides a look at a culturally specific practice that naturally incorporates a number of strategies used in positive psychology. Kwanzaa is an African American cultural holiday celebrating African cultural heritage by highlighting seven cultural principles or *Nguzo Saba* [52]:

1. *Umoja* (unity).
2. *Kujichagulia* (self-determination).
3. *Ujima* (collective work and responsibility).
4. *Ujamaa* (cooperative economics).
5. *Nia* (purpose).
6. *Kuumba* (creativity).
7. *Imani* (faith) [52].

During the Kwanzaa holiday (December 26–January 1), families and communities come together and spend time reflecting on the principle of the day, giving thanks to the ancestors for their examples of perseverance, expressing gratitude to one another for mutual support and enrichment, and identifying new ways in which one can exemplify each Kwanzaa principle in one's own life. African American patients may derive significant benefit from engaging in positive psychology activities using Kwanzaa as a frame of reference. Such an exploration need not be confined to the actual Kwanzaa holiday. While the specific practice of Kwanzaa may resonate most with people of African cultural heritage, patients of other cultural backgrounds may have similar celebrations around which they could build their own culturally relevant positive psychology practices.

Summary

In conclusion, as more clinicians come to appreciate the value of positive psychology and psychiatry principles, their ability to integrate these ideas into everyday practice is hindered by a variety of different techniques, methods, and practices that restrict their approach [1, 53]. As mental

health clinicians, our goal should be to provide every individual we serve with skills, tools, and corrective emotional experiences that enable them to achieve optimal human functioning. This is achieved by not only addressing their areas of deficit but also building up their strengths [4, 12, 30]. Also of central importance is self-reflection on the part of the clinician and a commitment to cultural competence [35, 38]. A positive psychology and psychiatry approach emphasizing and supporting adaptive strengths [9] and collectivist values in addition to the core constructs outlined by Seligman et al. holds great promise as a way to help address the mental health needs of multicultural populations and enable the provision of high-quality, culturally competent care [9–11].

This is of particular importance given the barriers minorities in the United States experience in accessing mental health-care services [5–7, 19]. Sriwattanakomen has outlined the necessary strategies to circumvent disparities in mental health care [7]. Further exploration into ways that positive psychology, psychotherapy, and psychiatry strategies can be incorporated into clinical care, community-based participatory research (CBPR) interventions, outreach programs, and community psychoeducation is essential [20].

Key Points

- Positive psychology, psychiatry, and psychotherapy add to our traditional mental health armamentarium by providing an understanding of the factors that enable people to flourish and by providing tools to assess and build individual strengths [2, 3, 12].
- Positive psychology's applicability to multicultural populations has been understudied. Yet, because its core constructs center on virtues shared by most cultures across the world [4] and because it emphasizes strengths over deficits, positive psychology has the potential to serve as a powerful foundation for high-quality, culturally relevant mental health interventions.
- Successfully implementing a culturally competent positive psychology approach in clinical care requires the provider to engage in self-examination around 5 key areas:
  1. Commitment to a strengths perspective in patient care [8, 30].
  2. Commitment to the five constructs of cultural competence [35].
  3. Ability to recognize, appreciate, and affirm the adaptive strengths [9] possessed by patients from marginalized communities.
  4. Commitment to eschew notions of the superiority of one cultural frame of reference or belief system over another.
  5. Commitment to shared decision-making [45].
- Positive psychology interventions that (1) acknowledge the effects of historical and ongoing systemic inequality, (2) explore and emphasize collectivistic values, and (3) recognize key adaptive strengths [9] have the potential to be powerful tools for cross-cultural mental health promotion [9, 11, 29].

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Positive Psychotherapy in Different Cultures

21

Enver Çesko and Ebru Tansel

Positive Psychotherapy and Transcultural Encounters

In the past, the majority of people lived in the same city in which they were born, but today, people can easily move from one city to another and even to different countries. Migration occurs for many different reasons around the world; some move to improve their lifestyles, whereas others seek better economic conditions, education, or migrate for certain political reasons. A transcultural meeting between two individuals is more possible today than it ever has been in history. This is why a transcultural understanding of clients is vital in therapeutic practice. In psychotherapy, transcultural issues are important to understand the client's point of view and what he really suffers from. Without this understanding, the therapist may make the wrong assessment and apply an unsuitable approach. However, it is also essential for the client to understand the transcultural issues related to his/her own life events. The tools and concepts of PPT help the client to gain an insight into the transcultural issues of his life more easily.

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Whereas physical complaints are regarded as focal illness in many Western cultures, by trying to investigate physical symptoms, in Eastern cultures, the problem can be understood by explaining the real reason for the causes of particular physical complaints. One client explained how he had to seek medical assistance for stomach pains after dinner at a friend's house. During the dinner he was repeatedly invited by the host to take more food and, not to be rude, he felt obliged to accept each time. From the traditional perspective of the host, it is expected to serve more food than required to show his respect and readiness to entertain his guest. From the different cultural point of view of the client, the politeness of the guest requires him not to leave food on the plate to show his appreciation of the host. As a result of his loyalty to his friend and his politeness, he suffered from a stomach ache and had to go to the emergency room. In PPT, this situation can be understood by explaining the meaning of loyalty and politeness. The matter is how both sides understand cultural habits in terms of time, environment, and between people to avoid misunderstandings.

In PPT, capacities represent expressive behaviors in terms of how people act in certain situations, with certain people and at certain times. Both primary and secondary capacities develop from early childhood as a result of internal and external influences of the family and subsequently become a social part of the community

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_21

trying to adapt to the norms, regulations, and patterns in society. By accepting and reacting according to the social norms, an individual has to balance his/her developed primary capacities from the family environment in social encounters. Often, this process of transformation from the narrow family, usually from the traditional patterns passed through different generations, makes it difficult to invest in and be accepted in new social and community environments. These difficulties create problems and misunderstandings, meaning that these conflicts first need to be understood and then solved [5].

Transcultural understanding includes ethnological, cultural, anthropological, and sociopolitical aspects of social development. Research has shown how ethno-psychotherapy [2, 9, 13, 15, 18] has an important impact on the success of the treatment process, whereas anthropological [37] studies have shown how the focus is on the cultural elements to achieve a greater understanding of social and human development. PPT considers all of these aspects from a transcultural perspective; it is important to understand the whole process from the beginning, in terms of how it was, how it is, and how it should be.

Interaction in PPT starts from the stage of attachment, when the therapist and patient meet each other and begin to understand each other through the presentation of complaints and needs, to the differentiation where they can work differently by using their own capacities, skills, and opportunities to achieve results. In this move from the starting point (attachment) to the stage of differentiation, which challenges the patient's own capacities for better results, the patient is becoming aware of his/her suppressed or unused capacities that he/she may activate to achieve successes in life, to reach new goals (detachment), and to become independent, helping not only him/herself but also to be an adviser for others.

How Positive Psychotherapy Started in Different Cultures "from West to East"

We are living in a time in which many transitions in living conditions are taking place. Today, the problems surrounding refugees and migrants are influencing the basic needs of human beings, such as food, environment, relations, and contacts, in addition to global health policies. The movement from one place to another presents different challenges in terms of cross-cultural adaptations.

Positive psychotherapy, with its broad consideration of different methods, techniques, and principles, is very suitable for different cultural environments. For this reason, the founder of PPT, Nossrat Peseschkian [28], first started his "journey" in his own clinic in Wiesbaden, Germany, working with patients from different cultures, using the stories from his cultural background and very often also presenting them to the patients from Western cultures. He also noticed that the patients from Western cultures found his storytelling to be beneficial, in addition to the patients from Eastern cultures.

As a result of his interest in developing PPT in other countries, Peseschkian started his journey by giving seminars, workshops, and trainings in some 68 countries.

Subsequently, his colleagues and co-workers continued to spread and develop PPT in their own and other countries. Many positive psychotherapists not only started to apply PPT with their clients in their own countries, but organized different kinds of training programs that promoted PPT as a transcultural and psychodynamic approach.

The application of PPT in different cultures has been examined and described in various publications [4–7, 12, 14, 17–19, 22, 33].

Positive Psychotherapy and Its Application in Different Transcultural Communities

Peseschkian grew up in Iran and received his medical education and specialization in Germany. The transcultural situation in his own life helped Peseschkian to realize how psychosocial norms and capacities are important in socialization and interpersonal conflicts. "Actual capacities are the behavioral patterns. Actual capacities are contents of education and they are taught to the children according to the needs of society" ([26], p. 45). Peseschkian emphasizes society to be the chief transmitter of the norms by which we appraise certain behaviors ([26], p. 105). For example, in an industrialized culture, more emphasis may be placed on secondary (behavioral) capacities such as punctuality and orderliness, whereas primary (emotional) capacities may be neglected. In one culture, a child who obeys his parents without discussing the meaning of their rules may be perceived to have a good attitude, whereas in another culture a child who questions what he is told might be appreciated. A young man who gives up a relationship with his girlfriend as his family does not approve may be perceived to have dependent personality features in a more individualistic culture, but in more collectivist cultures it is very important to get approval for marriage from the family, and this might be what he should do.

Actual capacities develop in a *certain situation* and at a *point in time*. During the psychotherapy of a university student, it was revealed that being successful at school, obtaining a scholarship and not becoming a financial burden to the family were particularly important for her. Her family had been forced to migrate during the war in Cyprus in 1974, causing them to leave all their possessions and they experienced financial difficulties in the place in which they settled. However, about 40 years after the war, at the time she was attending university, her family had already succeeded in recovering from these financial problems, although she remained very careful about her expenses. Even though times had changed and they had a better financial situation, frugality was still very important, and this created significant amounts of stress for her. To understand the function of an actual capacity. It is important to understand the context and time in which it was developed and sometimes this might be related to previous generations.

Standards may also change over time, which causes intergenerational conflicts. Today, the parents of many adolescents complain about how much time their teens spend on computer games on the internet, which has similarities to the complaints made by parents 30 years ago about how much time their teenagers spent watching TV and talking on the phone. If the grandparents engage in the raising of their grandchildren, conflicts can frequently emerge between the parents who follow the current guidelines from pediatricians about how to feed and look after the baby and the grandparents who believe that the best method is the one they used many years ago to look after their own children. As Peseschkian says, the antidote to overcoming intergenerational conflicts may be to learn how to unite the past, present, and future ([26], p. 112).

In his book "Positive Psychotherapy in Psychosomatic Medicine" [31], Peseschkian demonstrated how two essential aspects of human nature, the concept of body and mind, can be unified. The method of his paradigm is that these two concepts can work together and share common factors by influencing each other. In the field of psychosomatic medicine, body and mind (sometimes identified as the soul), are always working in tandem, because the words *"psych-o"* and *"soma-tic"* are related to the mind and body. Furthermore, this work demonstrates that human beings are good by nature, and that all individuals have the power and capacity to take care of their own wellbeing. For this reason, he believed that a person, with his or her cognitive and spiritual being, has an unlimited capacity to reach his or her goals. All difficulties, disorders, illnesses, and problems, even aspects of the mind, manifest through psychosomatic symptoms, usually starting in the body and explained through the mind (cognitive expression).

Many examples presented in the book [31], show how a variety of disorders may be treated

from the transcultural aspects through the use of different tools that patients from both Western and Eastern cultures can easily understand and accept.

The personal experiences of trainers working in Eastern and Western Europe, the Middle East, the Balkan countries, African cultures, and in America show that PPT is very warmly and closely accepted because of its wide semi-structured methodology in creating the relationship between the client and therapist. For example, using stories all the time and without any exceptions had a special stimulus when attempting to find similar solutions, as this allowed the clients to solve actual problems, difficulties, and symptoms. The therapist presents a different concept via the story, which gives the patient an alternative way of understanding or solving his/her problem. "Identification with this new point of view enables the patient to get a better understanding about his/her situation and a different way of coping with the problem" ([24], p. 264).

Positive psychotherapy offers a transcultural perspective in the form of stories, proverbs, myths, and fables in which the client/patient may recognize him/herself in "allegorical terms," and is therefore able to establish a new relationship between the messages received from the story and his/her actual complaints. Stories are predominantly used during the psychotherapy sessions for two reasons: one is to stimulate the patient's creativity and self-awareness to find similarities between the patient's life story and the story being told; and the second is that the patient can use the ideas taken from the message story to solve his/her own problems. Also, the stories are very powerful for reflecting the reader's own situation in psychotherapy education settings as a method of self-exploration and self-discovery with the students [27].

The use of three principles known as

1. The Principle of Hope, with a positive interpretation of actual complaints.
2. The Principle of the Balance Model, as meta theory in making the awareness of human capacities.
3. The Principle of five stages in therapeutic treatment enables PPT to be adapted to different cultures without difficulties.

The *model dimensions* of PPT help to clarify the concepts that are valid in the family of origin. The individual's attitude toward him/herself, expectations in romantic relationships, the way of making contact with others, and his world view are all shaped through his experiences with others who serve as models. During the sessions of a couple who applied for marital counseling, the wife complained about her husband's indifference. She claimed that he would never make a comment about her clothes, friends, or activities. The man was surprised to hear this. When the model dimensions of each were discussed, the couple could see that they were both from conservative families in which the father was the dominant figure. As a child, the husband always believed his father interfered too much with his mother and disturbed her with his endless comments about what she should or should not do. As a small child, he promised himself that he would never act like his father when he became a husband. He never made a comment about his wife's clothes, friends, or hobbies as he believed she had the right to choose them herself. The wife talked about her parents' relationship in an idealized way, as a loving relationship. The father had been very concerned about his wife, would buy her clothes himself, and they would spend much of their time together. Her father had died when she was 12 years old. She had an idealized view about her father and her parents' relationship, although it sounded very oppressive.

Another tool of PPT is the concept of the *family tree*. The concepts of the family are not only formed by the family members' experiences but also from the experiences of the past generations and political and philosophical traditions of the family. These concepts shape the rules of our way of life, our interaction with others, and the way in which we attempt to resolve conflicts. Peseschkian suggests that "to understand a family's situation, the therapist should learn about the situation of the parents' family of origin" ([24], p. 300).

Developing an understanding about the content of his/her conflict enables the patient to solve his problems or be less affected by them. The patient becomes a therapist for himself and for others around him.

Transcultural Concepts in Positive Psychotherapy in Comparison with Other Psychotherapy Modalities

As PPT belongs to the psychodynamic and humanistic approach, one of the goals of therapy is the remission of symptoms, where the success of treatment is not only based on relieving the symptoms but also on fostering the positive presence of psychological capacities and resources [35]. As stated above, the founder of positive psychotherapy, Nossrat Peseschkian, was originally from Iran and moved to Germany to start work as a neurologist, psychiatrist, and psychotherapist. He used concepts derived from Eastern culture and applied them to his patients from Western cultures. He was inspired in the 1960s and 1970s by his personal contacts with eminent personalities in the fields of humanistic psychology, psychiatry, and psychotherapy, such as Victor Frankl, Jacob L. Moreno, Raymond Battegay, Gaetano Benedetti, Heinrich Meng, and others.

Compared with other psychotherapeutic methods, where the core focus is on analytical and behavioral drivers that can be interpreted as different stimulus responses of individual reactions (psychoanalytic and behavioral approaches), PPT focuses on understanding and increasing the awareness of personal capacities and how they are developed in relationships with family and the social environment [24].

"For its part, positive psychotherapy defends the view that environmental influences continually affect the individual, and that prior experiences, in the sense of micro traumas, form the frame of reference for subsequent experiences. Not only early childhood but each and every period of development has psychological effects" ([24], p. 379).

As a meta-theory, PPT sometimes uses techniques and tools of other psychotherapeutic schools, such as behavior therapy, individual psychology, analytical psychology, logotherapy, gestalt therapy, primal therapy, transactional analysis, at the same time, PPT is a wider concept. As *logotherapy* emphasizes the meaning of life, PPT includes logotherapeutic elements at different stages of treatment and actual capacities. From *gestalt therapy*, in which the focus is on the solution of present problems, PPT uses "actual capacities and experiences that a person has had within his environment" ([24], p. 396).

The fundamental concepts that *psychoanalysis* includes, i.e., the role of the unconscious and the structure of Id, Ego, and Superego, are also common in the majority of other approaches. PPT combines these three concepts with four aspects of life using the balance model (body, achievement, contacts, future), the model dimension (I, You, We, and Primary We), and three levels of interaction in therapeutic relationship (attachment, differentiation, and detachment).

In *behavioral therapy*, symptoms are treated by changing behaviors. PPT underlines the role of actual capacities, where patients are using the tool DAI to have a broader clinical profile and identifying the conflicts between partners.

Transactional analysis uses three ego states (parental, adult, and child) as the interaction between two persons, whereas PPT considers these three ego states, especially in group family therapy and the context of situational encouragement.

Body Psychotherapy and Positive Psychotherapy, both with psychodynamic character in their methodology treatment are using semi-structural approach where the main focus is the client as a complete being in body, mind, feelings and spirit, motivating the use of actual capacities to achieve the best quality of life [6].

As a psychodynamic and humanistic approach, many other schools and modalities can be integrated into the PPT treatment system. "Positive psychotherapy itself is not to be understood as an exclusive system, but rather attributes a particular value to each of the values of psychotherapeutic methods. Therefore, psychoanalytic, depth

psychology, behavioral therapy, group therapy, hypnotherapy, psychopharmacology, and physiotherapy are considered. Positive psychotherapy thus represents an integral method in the sense of a multidimensional therapy" ([24], p. 400).

Individual lifestyle and acting patterns in daily life in terms of various concepts, habits, behaviors, and achievements produce many conflicts—often as a result of extreme forms of different kinds of orientations that are associated with individual capacities.

Therefore, the transcultural approach is like "a red thread" in the psychotherapy process, "because the transcultural aspect also offers material to understand the individual conflicts" ([29], p. 102) between the client and society, and all other significant aspects in life.

Transcultural Understanding of Capacities from the Point of Positive Psychotherapy

The general understanding of Positive Psychotherapy method is that every human being is "good by nature", and this is based on two main capacities: capacity to love and capacity to know. According to these two basic capacities, every human being is developing relationships among each other. But all these relationships are based in understanding of actual capacities developed through everyday life circumstances and they "are developmental dimensions, the shaping of which is furthered or suppressed by favourable or inhibitory environmental influences" ([28], p. 43). Capacities from the point of Positive Psychotherapy are understood as changeable and developmental categories that are not static and unique to all human beings, they can assess differently according to situation, experiences, manners, attitudes, and personality structures. While primary capabilities are related to the emotional aspects of humans, secondary capabilities are related to behavioral aspects [22].

Primary and secondary capacities determine themselves. Both the overdevelopment and underdevelopment of secondary capacities constitute the source of important problems. The underdevelopment of ones shows that individuals cannot demonstrate their capacities in the relevant field. Highly developed capacities, on the other hand, are the individuals' use of their energy at a high level and in a non-functional way related to that field. Acting with too much energy also causes the individual to fail. Therefore, using capacities in a balanced way is important for physical and mental health [26].

The individual cultural heritage is a major determinant factor of mental health issues where cultural values system with cultural beliefs, religious and spirituality with a "Lack of awareness of important cultural differences can undermine the development of a therapeutic alliance and the negotiations and delivery of effective treatment" ([8] p. 69).

The capacities of human beings from the transcultural point of understanding may also be seen as behavioral reactions among the people in different situations. This means a psychodynamic follow-up to understand conflicts of disbalance in four areas and misunderstanding of model dimensions in development of primary and secondary capacities. It is research done in different cultural populations that showed how people are showing specific capacities in specific situations, with different people and in different circumstances. And all these capacities are named as actual capacities [21]. According to some authors the actual capacities are specified as primary actual capacities and secondary actual capacities, where the first one is a matter of structural functions of personality, and the second one is based on skills and behavioral standards [19, 20].

The classical approach to psychotherapy is the focus centered on the disease, while the approach in positive psychotherapy keeps the focus on the client and his abilities. This model of treatment means the transcultural approach in the development and understanding of primary and secondary abilities creating the structure of the human personality. Therefore, in different cultures, actual capacities might have different meanings and values that can cause misunderstandings, difficulties, problems, and often conflicts in interpersonal relationships. That's why the actual

capacities affect our personal and social attitudes, judgments, and values [32].

As an example, Positive Psychotherapy considers honesty within the scope of secondary capacities. In this context, honesty means that individuals are open and telling the truth. When individuals believe that *"openness and honesty are more important than being respectful to others"*, in some cultures, this means; *"people who express their opinions in an extremely open and direct way",* and when they 'choose to express their opinions openly', it means that they use their honesty skills at a high level. These different expressions may cause misunderstandings and produce the risk for conflicts and react with frustrations or aggressions.

In positive psychotherapy, the capability of obedience is evaluated within the scope of secondary capacities. In this context, individuals say, *"I follow the guidance and advice of a boss or a person in authority," "Children should always care about what their parents say and obey them."* But in some other cultures the most comprehensive concept of these is compliance. Compliance refers to the change in behavior or belief as a result of real or imaginary group pressure. Compliance means thinking and behaving differently than when we are alone. *"I may agree with you, even if I have my personal opinion"*.

Primary and secondary capabilities are developed as individuals interact with the physical and social environment within a certain process [22]. Socialization processes are important in the development of primary and secondary capabilities. At this point, the importance of relationships with parents in the development of capabilities cannot be denied [28]. At the end of the socialization process, human capabilities determine themselves as a part of the personality. Thus, primary and secondary capabilities affect our personal and social attitudes, judgments, and values [32]. Disruptions in the development of abilities as a part of the personality cause the emergence of mental disorders [23].

In one research done with Turkish population, focusing on the relationship between PTSD symptoms and primary and secondary capacities, it was found that severity of PTSD symptoms had no significant correlation with secondary capacities but had significant negative correlation with primary capacities and this may be because the primary capacities are more related to emotional needs and relations [34]. It is obvious that socialization is an important process in the development of primary and secondary capacities and the crucial point for healthy development of relationships among people is the early stages of relationship between parents and children where both primary and secondary capacities are developing. That's why primary and secondary capacities influence our personal and social attitudes, judgments, and values [22], and if there are any obstacles or defects in the development of capacities in personality can produce mental health disorders [23].

The influence of culture in the development of psychotherapy as a holistic approach, where Positive Psychotherapy belongs to this approach, the human being is seeing as a whole. But nevertheless, some new developments in the society impact the structure of family constellation where collective identification remained primary, when a person feels firstly a member of a family and society where independent thinking and independence are discouraged from childhood and bring up obedience, collective thinking, self-identification as a member of the family and society [11].

That's why some values of authoritarian and collective society are different from the values of more individualistic modern western society [10, 15].

Differences between Western culture where the individual is seeing as a psychological being focusing in development of capacities for independent functioning, in the East culture, individual is seeing as social being where his development is on growth of social functioning; creating a family, getting work, being accepted and respected in his society [3, 9].

We psychotherapists, during our work, are meeting in our offices these two different cultural backgrounds, where clients and psychotherapists who have different cultural values, concepts, beliefs, and capacities, can produce a lot of misunderstandings and mistakes [36]. To understand

these cultural differences, psychotherapists need to be familiar with clients' cultural educational development [36] where the actual capacities are developed.

Positive and Transcultural Psychotherapy and Global Development

Today, we are living in very unpredictable, stressful, and insecure environments, which can make it a challenge to reach our life goals. The time in which we are living is witnessing constant developments in terms of science and technology, associated with extreme materialistic aspirations. A large proportion of people in society consider themselves to be believers who are utilizing spirituality as important aspects of their lifestyles. Clients and therapists often have considerable differences in their views, concepts, habits, and behaviors, and how they approach problems, difficulties, disorders, and diseases. Usually, one side in this process is not enough in consideration of the cultural background of the other side.

Good psychotherapy requires proper understanding of the patient's culture. The cultural norms of the patient, his/her religious beliefs, concepts of disease, ways of expressing emotions, language, attitudes, and social norms should be considered to understand the patient [1]. The psychotherapist should not also forget the impact of his own cultural background on his psychotherapy work. The therapist's own cultural background can create a blind spot in evaluating the patient. The better the therapist knows himself, the most likely it will be to overcome cultural prejudices.

Positive psychotherapy is based on a psychodynamic and humanistic approach that gives emphasis to transcultural issues. It is a resource-oriented and conflict-centered method where symptoms are unsolved conflicts derived from the past, usually from childhood. In PPT, the term "positive" comes from the Latin word "positum," which means factual and given; and allows the therapist to see the client as a whole personality, where he/she is able to manage the problems, challenges, disorders, and diseases [27]. The basic concepts of PPT can be phrased in everyday language and this makes it easy to be understood by different cultures, ethnic groups, and communities during the therapeutic process in the relationship between the therapist and patient. Transcultural psychotherapy with its basic concepts provides a framework of the psychotherapeutic process, where communication between therapist and client with different cultural entities can be promoted.

If a therapist originates from a traditional and religious background, when contacting, lecturing, presenting, and introducing the basic concepts either for students or for clients, he has to respect and accept all existing different cultural norms.

When a person from Eastern culture meets a friend from Western culture, and if his friend asks, "*How are you?"* he will not feel good, because he is not also asked about his family. This is because, from his perspective, family is more important than the person alone. But, if he meets a friend from Western culture, and asks, *"How are you?"* the question automatically elicits the reply; *"Fine, thanks. And you?"*

This shows how different cultural values and norms assess different reactions and behaviors from people who assign importance to different concepts and norms. For one person, how he or she is developing with regard to career and body, physical form might be important (usually for Western cultures); for somebody else, the relationship, contacts, and spiritual values might have more importance (usually in Eastern cultures).

Social changes are rapidly moving from one extreme to another. This rapid change is perceived consciously only to a limited degree [26], and the norms and goals of yesterday may suddenly become questionable and a burden today. One of the most stressful challenges during the last 20 years has been trying to maintain this balance.

The world is facing increased population growth, which is becoming one of the greatest challenges in society, creating problems in terms of food, socioeconomic forces, migrants

and refugees, terrorism, etc. This creates the problem of urbanization as an indirect result of the population growth, as a result of migrant and refugee movements in the search for better socioeconomic conditions. The problem of urbanization brings new influences in the division of labor, where mankind is moving from differentiation to specialization in the sense that roles are constantly changing. This might be observed more easily in cultures that are still in political and sociological transition, and trying to find a real identity. Many families under the influence of global changes are attempting to find a new identity by losing their traditional family structures. And finally, Peseschkian points out that these changes make many national, ethnic, and cultural groups have more contact with outside groups today. This may create new possibilities and new transcultural problems [26].

Peseschkian identifies different values and norms, not only between people from different cultures, but also between parents and children (intergenerational conflicts), between marriage partners, and among members of other groups. A transcultural way of thinking must consider not only the major cultures, but also the subcultures, which have their own standards. People tend to forget that there are norms and values other than their own and this causes misunderstandings ([32], pp. 49–51). When two people get married, even if they are from the same village, living on the same street, this is still the meeting of two different cultures. Each family has its own way of living. In one family, dinner should be eaten at the same time and with the whole family sitting at the dinner table, whereas in the family of origin of the other spouse, dinner might be eaten by the family members separately at different times. Actual capacities may be the source of misunderstandings. If "punctuality" is very important for one family, the spouse who comes home late for dinner may be perceived as disrespectful. Or, if "contact" is very important, it might be perceived as rude if the other family members do not wait for each other to eat dinner. Patience and contact become more important virtues than punctuality.

Case Example

A couple had applied for marital therapy. The main complaint of the woman was her husband's unceasing financial support for his family of origin. The husband was the eldest son of the family and he would provide financial support to his brothers and sisters. His wife had accepted her husband helping his siblings during their education and marriage, but even though they were now adults, they would still ask for help when buying a house or starting a new job. She believed that the siblings were manipulating her husband. The discomfort of the woman was obvious; she wanted the family budget to be used for themselves and for their children's needs. From the husband's perspective, he was not being deceived by his siblings, but his decision to support them was his choice. During the therapy, it was understood that his sacrifices were rewarded with respect and occupying the dominant position of the father of the extended family. The man wanted to remain a part of his extended family and the woman wanted to be a nuclear family. Both family types have their advantages and disadvantages. An extended family provides affection and protection, but also demands integration and contact ([32], p. 56).

Psychotherapeutic treatment must be chosen on the basis of the client's cultural background to be able to help the patient with the aspects of cultural adjustment. It is important for modern psychotherapists who are working in mixed ethnicities and with different cultural backgrounds to know the qualities necessary for culturally competent psychotherapy. The modern psychotherapist needs to develop special qualities such as *cultural sensitivity*, *cultural knowledge*, *cultural empathy*, and *cultural insight* [36].

Therefore, in general, positive and transcultural psychotherapy have to deal with two main questions, which are important for maintaining the harmonious connection between different cultures: the first is *What do all people have in common,* and the second is, *How do they differ*? This can be better understood if human beings are aware of and able to accept the similarities where they can meet, but not when they are far

from each other. Different worlds of traditions clash because of their diverse content and goal projections and the question always arises: how well can these worlds harmonize under the new conditions? [31].

Conclusion

Positive psychotherapy is based on a psychodynamic and humanistic approach that places emphasis on transcultural issues. The cultural point of view of the individual affects how he/she appraises certain behaviors and events in his/her life. Sometimes, in interaction among themselves, people tend to forget that different norms and values other than their own exist and this causes misunderstandings and conflicts.

The meeting of the client and the therapist is also a transcultural encounter. The therapist should gain a good understanding of the client's cultural background to make an accurate assessment and an effective therapy plan.

It is also essential for the client to understand the transcultural content behind the symptoms and interpersonal conflicts. Concepts of PPT can be phrased in everyday language and provide practical tools in the therapy process. Investigating one's actual capacities, model dimensions, and family concepts helps the client to understand what is underpinning the conflicts.

Being aware of their own capacities, the patient and therapist, in PPT settings, explore common elements why conflicts, problems, and illnesses occur at certain times, in certain environments, and in the body. Case vignettes are given within the text to clarify how these are used in practice.

Different values may exist, not only between people from different cultures but also between parents and children (intergenerational conflicts) or between partners. A transcultural perspective helps understanding not only interpersonal problems but also relations among people in different cultural communities. Positive and transcultural psychotherapy deal with the operationalization of two sides: the Eastern and the Western approach, which are both important for maintaining a harmonious connection between different cultures. This is especially relevant in the world today.

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Positive Sports Psychiatry

22

Francis Aguilar and Garrett Rossi

Introduction

As physicians, our essential role is to identify health disparities and at most levels meet our patients at the point of tertiary treatment. The specialty of psychiatry embraces and executes medical decisions based on a model that evaluates a patient's biopsychosocial formulation in order to manage and treat psychopathologies. The implementation of positive psychiatry focuses on well-being through a variety of methods. Positive psychiatry seeks to understand well-being and how to identify and enhance overall health through biological, psychological, behavioral, and psychosocial interventions. Its applications extend past recognizing overall well-being and can be implemented to identify performance enhancement. An area where the principles of positive psychiatry may be of value is the emerging field of sports psychiatry. Sports psychiatry aims to deliver and optimize psychiatric care for athletes of all levels. This specialized field looks to identify psychological traits (resilience, optimism, personal mastery and coping self-efficacy, social engagement, spirituality and religiosity, and wisdom including compassion) and environmental factors (family dynamics, social support, and other environmental determinants of overall health) and understand the neurobiology/neurophysiology of stress related to sports. Sports psychiatry and positive psychiatry share the common goal of enhanced well-being leading to improved performance in many aspects of a patient's lives. Both disciplines focus on thought process and psychological skills such as goal setting, visualization, and relaxation techniques with the goal of enhancing overall well-being.

Inside the Athlete's Mind

There is no specific trait that defines a person as an athlete. However, there are some general personality traits that are somewhat pervasive in the athletic community. People who compete in athletics tend to be more extraverted and exhibit high degrees of perfectionism and narcissism. These personality traits while not pathological in themselves do predispose athletes to certain psychopathologies. Athletes tend to develop a narrow range of focus early in their careers. This can be a benefit to them when developing a skill in a sport and achieving athletic success. The intense focus on performance and achievement in sport can interfere with maturation and development particularly in young athletes. Success in athletic endeavors does not always translate to success in other areas

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_22

of life. The lack of maturity and psychological development outside the area of sport places athletes at risk for mental illness. Athletes are at high risk for "burnout" and overuse injuries which can further predispose them to psychiatric disorders. Given the high levels of confidence, combined with narcissistic and perfectionist traits, athletic mindsets pose a unique challenge to the sports psychiatrist. The risk of receiving suboptimal care exists for the athletic population. As athletes gain more notoriety, they receive higher status in comparison to peers. Furthermore, there are unique ethical issues faced when treating this population. For example, professional boundaries and confidentiality are at risk of being violated if the clinician is not mindful of professional standards.

The Link Between Athletes and Mental Illness

People who achieve a high degree of success in athletics are viewed as the pinnacle of health and well-being. The general population perceives these people as healthy, physically fit, and mentally tough. Research indicates that athletes are not immune to mental health issues and prevalence rates for some mental illnesses are similar to the general population. The best studied disorders include eating disorders and substance use disorders. This population faces the unique risk of performance-enhancing drug use which includes anabolic steroids. Mental illnesses with the highest prevalence in the general population are relatively common among athletes. Some of these illnesses include depression, bipolar disorder, anxiety disorders, and attention deficit hyperactivity disorder (ADHD).

One study looked at the mental health of 224 elite Australian athletes (118 female, 106 male), which revealed 46.4% of athletes were experiencing symptoms of at least one mental health issue [1]. The study found percentages meeting criteria for mental disorders were similar to other studies of international athletes and community samples. Common psychiatric disorders included depression, eating disorders, and generalized anxiety disorder. Athletes who were injured had higher levels of both depression and generalized anxiety disorder symptoms. The authors concluded mental health problems reported by elite athletes appear similar to those observed in the community samples. In particular, injured athletes should be carefully monitored by a mental health professional given their increased vulnerability to depression and anxiety disorders.

Another study looked at a sample of high-level French athletes to determine variations in the prevalence of psychological problems based on sex and sport practiced. A representative sample of 13% of the French athlete population was obtained. Seventeen percent of athletes were identified as having at least one ongoing or recent disorder with generalized anxiety disorder being most prevalent. Overall 20% of women and 15% of men had at least one psychopathology [2]. The authors concluded that sex-based differences in psychopathology resemble those of the general population. Psychological stressors should be addressed early to help avoid further development of mental disorders.

Studies on the prevalence of psychiatric disorders among athletes are complicated. The samples are limited because of the lack of people who identify themselves as high-level athletes. There is often a lack of awareness or unwillingness to acknowledge the presence of mental health disorders on the part of the athlete and coaches. The athletic mindset of "toughing it out" can lead to a lack of openness and reluctance to seek help. There is still a stigma surrounding mental illness in athletes with high-level sports performance, even beyond what is seen in the general population. Early identification of at-risk athletes and connection with professional help can improve well-being, resilience, and performance in athletes.

Common Psychiatric Disorders in Athletes

Depression in Athletes

There is existing data in the literature indicating the benefits of physical activity on mood and depression. However, athletes can still be affected by depression. Athletes are fine discussing physical injuries, but emotional injuries are much more

difficult to talk about. The athlete does not want to appear to lack the necessary "mental toughness" to face the demands of the sport or be labeled as "a head case." As a result, many athletes go undiagnosed and do not receive the treatment necessary to improve their mood symptoms.

As detailed earlier in the chapter, there is difficulty in finding high-quality data regarding the prevalence of depression in athletes. Among college athletes the prevalence of depression is estimated to be between 15.6% and 21% [3]. Based on these rates, up to one in five college-level athletes is suffering from depression. Consistent with data from nonathletes, female athletes had higher rates of depression than male athletes. Other risk factors identified for the development of depression included being a freshman and having an injury or pain.

Athletes have unique risk factors for depression including the competitive nature of sport, high stress about performance, and burnout from overtraining [4]. As clinicians we cannot forget that athletes also have personal lives outside of sport and include similar stressors faced by the general population. There can be relationship issues with significant others, family issues, and financial issues adding to the demands of athletic performance.

Another unique risk factor for depression in athletes is injury. Injury is a part of playing sports, and sometimes severe injuries occur and have long recovery periods. These injuries can play a significant role in the athlete's mood, and the symptoms can be severe enough to cause posttraumatic stress disorder (PTSD)—like features such as reliving the injury through intrusive thoughts or nightmares. These athletes need to be closely monitored for the development of depression, and sometimes a psychiatrist will be called in to help facilitate the recovery process.

Depressive symptoms can present in unique ways and may be identified by coaches or trainers based on the athlete's performance. A depressed athlete may have a decreased interest in attending practice and look fatigued or out of it during games or overly self-critical of mistakes. These things can be mistaken as issues with the technical aspect of the sport or performance. Psychiatrists must recognize these signs and evaluate the athlete for depression.

Anxiety Disorders in Athletes

Anxiety disorders are poorly studied in this population, but one type of anxiety that is common among athletes is performance anxiety [5]. Athletes have a heightened level of anticipation prior to performing that does not interfere with other areas of their life. Performance anxiety is a common chief complaint among athletes seeing a sports psychiatrist. Determining ways to cope with the anxiety and reduce it to manageable levels that do not interfere with performance is a primary goal of treatment.

ADHD Among Athletes

ADHD is a common diagnosis in the general population with a prevalence rate of 7–11% among school-aged children. As a result, there will be a fair number of athletes that have the disorder, and there is possibly an increased prevalence of ADHD among young athletes. Physical activity and athletics can serve as a form of treatment helping the person to cope with the disorder. Athletes with ADHD may find certain aspects of learning a sport difficult such as learning the play book or certain rules related to the sport. Inability to focus or being easily distracted can affect performance. Stimulant medications are the first-line treatment in many cases for ADHD. This poses unique issues for athletes as these medications are considered performance-enhancing drugs. They often require a therapeutic exemption and can be a barrier to adequate treatment of the disorder in this population. The diagnosis can also add to self-esteem issues which also affects performance.

Eating Disorders

Many athletic endeavors require athletes to be in a certain weight class in order to compete. There is a concept in sports known as "weight cutting"

where an athlete will choose to compete at a lower weight class than their natural weight. In order to achieve this goal, they undergo strict dietary restriction and often various dehydration protocols to achieve this goal. This behavior, along with the desire to achieve an athletic advantage, can lead to eating disorders. The opposite can also be true where athletes are encouraged to add additional weight to their body in sports where large size is a competitive advantage. It's no surprise that the most studied psychiatric disorders among athletes are eating disorders.

One study looked at the prevalence of eating disorders in Norwegian elite athletes both male and female in comparison to the general population. The study found that there are higher rates of eating disorders in elite athletes (13.5%) compared to controls (4.6%) [6]. They also found that female athletes were more likely than male athletes to have eating disorders. Female athletes are at an increased risk of serious medical complications from severe caloric restriction including chronic fatigue, menstrual dysfunction including amenorrhea, and low bone mineral density that could lead to osteoporosis [7]. This is most common in sports that emphasize leanness and appearance, such as wrestling.

Most athletes do not meet full criteria for a specific eating disorder; rather they have unhealthy practices with regard to eating. The diagnosis that is commonly applied to these athletes is unspecified feeding or eating disorder. This can present in a variety of ways, and athletes should be screened for unhealthy eating habits. Although eating disorders mostly affect female athletes, it's important to screen both males and females for this. Risk factors in both groups include a family history of eating disorders, a personal history of eating disorders, female gender, and mood or anxiety disorders.

Substance Use Disorders

The National Collegiate Athletic Association (NCAA) conducted a study of substance use habits in college athletes using a self-reported retrospective survey. Alcohol was found to be the most widely used drug in the past 12 months (80.5%), followed by cannabis (38.4%) [8] and smokeless tobacco (22.5%). The study indicated that substance use was highest among Division III athletes and among Caucasians.

Athletes that use substances such as alcohol have decreased athletic performance as well as negative health consequences. Alcohol is the most commonly used substance by athletes and is often cited as a cause of reduced performance. Athletes are at increased risk of problematic alcohol consumption and have higher rates of binge drinking. Alcohol can affect performance by disrupting sleep patterns, reducing inhibition leading to more impulsive and dangerous behavior. Screening tools can be used to clarify the diagnosis and assess severity of substance use disorders.

Performance-enhancing drugs are a common problem in sports. Drugs such as anabolic steroids, amphetamines, human growth hormone, and erythropoietin provide a competitive advantage in sports. These substances are banned at all levels of athletic competition. Most athletes are subjected to random drug testing to help ensure a level playing field. In some cases, the use of performance-enhancing drugs can lead to psychiatric symptoms. Increased aggression is a well-established side effect of anabolic steroid use [9]. Anabolic steroids have also been known to cause mood symptoms. Amphetamines often used to increase focus and attention can increase irritability, lead to poor sleep, and decrease appetite. The added pressure of winning allows an athlete to want to have a perceived advantage by using these drugs.

Psychiatric Treatment of Athletes

There are two primary methods for treating athletes suffering from psychiatric illness: pharmacotherapy and psychotherapy. Psychotropic medications are known to have adverse side effects that may not be well tolerated by athletes. Most antidepressants have the risk of developing sedation, weight gain, and gastrointestinal dis-

tress that would be counterproductive for athletic performance. Athletes may be reluctant to take these medications if they believe it will interfere with their ability to perform. Stimulant medications used to treat ADHD will require a therapeutic exemption given the performance-enhancing abilities of these drugs. Patients being treated with benzodiazepines for anxiety or panic disorder will test positive on typical toxicology reports which can result in disciplinary action against the athlete. It's essential that the clinician discuss with the patient the risks and benefits of pharmacological intervention so the athlete can make an informed choice.

The other primary method of treating athletes is psychotherapy. The goal of this intervention is to alleviate psychiatric symptoms and improve athletic performance. The sports psychiatrist should be well versed in traditional "sports psychology" that focuses on goal setting, self-talk, relaxation techniques, and guided imagery. These techniques focus more on the performance aspects of athletic pursuits and less on the treatment of specific psychiatric disorders. The goal of talk therapy for treatment of psychiatric disorders is to alleviate symptoms with the goal of restoring optimal performance. Cognitive behavioral therapy (CBT) has a well-established record of reducing psychiatric symptoms in a wide variety of conditions. It is time limited and manual based, thus making it an ideal choice for the busy athlete. Most other forms of psychotherapy can be difficult to execute in the athletic population and have limited evidence to support their use.

The concepts of courage, optimism, honesty, and perseverance are important to develop for athletics. This is a focus of positive psychiatry, and enhancing these areas can lead to improved athletic performance. The sports psychiatrist should use a biopsychosocial formulation to identify key psychological traits (resilience, optimism, mastery, self-efficacy, and social engagement), and environmental factors (family dynamics, social support) as they relate to the neurobiology of stress in sport. Helping athletes to develop and utilize these traits leads to improvement in sport and life.

Applications

How can positive psychiatry be used in the treatment of athletes? Certain characteristics such as resilience and optimism not only play a role in overall well-being; they also have the potential to increase performance. Assessment of these characteristics and implementation of strategies to enhance these qualities should be a part of a sports psychiatrist's clinical practice. These practices would not replace current evidence-based treatments for psychiatric illness but can be used as an adjunct therapy to support or enhance current treatment. Practicing optimism, doing yoga, and daily mindfulness meditation practices can enhance well-being and performance. Patients should be encouraged to practice such lifestyle interventions to improve and strengthen these characteristics.

Positive psychiatry principles can be incorporated into any behavioral or psychosocial interventions. The goal is the same regardless of other interventions: to reduce symptoms and prevent relapse. To date, most psychiatric interventions focus on secondary and tertiary prevention. In contrast, positive psychiatry focuses on primary prevention. The goal of primary prevention is to prevent the onset of psychiatric illness and to strengthen key personality traits that foster resilience [10]. Resilience is associated with better health outcomes, lower risk of all-cause mortality, and overall increased life expectancy. In the high-stress world of competitive athletics, there needs to be a strong focus on therapies that enhance this characteristic.

Positive psychiatry interventions can be implemented immediately to complement any existing medication or psychotherapy intervention. The goal of adding this adjunctive measure is to help patients who have had partial responses and to prevent relapse of symptoms. Trails of interventions such as meditation, writing gratitude letters, practicing optimistic thinking, acts of kindness, and forgiveness therapy have worked well with other psychiatric populations. The change in attitude along with enhanced optimism has reduced alcohol consumption in people struggling with substance use and alters attitudes about aging in older adults. Optimism is a vital characteristic for athletes to develop and enhance. Having an atti-

tude of positive expectation can result in enhanced performance on the field and overall better quality of life off the field [11].

The potentials for biological interventions based on positive psychiatry are still being developed. As we increase our understanding of neuroplasticity and the genetics associated with these positive traits, pharmacological interventions to enhance adaptive function may develop. Areas of interest in biological psychiatry include enhanced adaptive function of the hypothalamic-pituitary-adrenal axis as well as monoamine, neuropeptide, and other stress response mechanisms. Given the limitation and testing for performance-enhancing drugs in competitive sport, these agents, if developed, will require ethical and legal review prior to being used in the athletic population.

The concepts of healthy diet, mindfulness meditation practice, gratitude journals, and therapy to improve resilience and optimism can be added to existing treatment plans immediately. While the shift from management of chronic psychiatric illness to primary prevention of disease processes is a goal of positive psychiatry, the goal of all medical and psychological interventions should continue to be to treat and cure disease if possible. Interventions should also maximize performance and quality of life. In sport performance is key, and any intervention and prevention that can improve and sustain resilience, optimism, and performance is likely to help ward off the onset or exacerbation of mental illness.

Summary

Both sports psychiatry and positive psychiatry are relatively new branches of general psychiatry. Athletes, although generally seen as more confident and physically fit than the general population, are not immune to psychiatric illness. The demands of competition can increase stress levels leading to burnout and reduced performance. In general, psychotropic medications are not well tolerated in the athletic population, and the adverse side effects of these drugs limit their potential use in this population. Positive psychiatry and sports psychiatry are not mutually exclusive and can enhance each other. Many of the benefits of positive mental attitude, stress reduction, and visualization are already components of high performance in athletics. As we learn more about the neurobiology of stress and development of resilience, more clinical applications will emerge. As more research develops, the focus can shift from management of mental illness to primary prevention in this population.

Key Points

- Sports psychiatry is an emerging subspecialty of general psychiatry with unique challenges.
- Confident, hardworking, and resilient athletes are still susceptible to prevalent psychiatric disorders.
- Common psychiatric disorders among athletes include depression, eating disorders, and generalized anxiety disorder.
- Early identification of at-risk athletes, and connection with professional help, can improve well-being, resilience, and performance in athletes.
- There are two primary methods for treating athletes suffering from psychiatric illness: pharmacotherapy and psychotherapy.
- Psychotropic medications are known to have adverse side effects that may not be well tolerated by athletes.
- Although cognitive behavioral therapy (CBT) has a well-established record of reducing psychiatric symptoms in a wide variety of conditions, positive psychiatry can enhance an athlete's well-being.
- The sports psychiatrist should use a biopsychosocial formulation to identify key psychological traits (resilience, optimism, mastery, self-efficacy, and social engagement), and environmental factors (family dynamics, social support) as they relate to the neurobiology of stress in sport.

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Positive Family and Marital Therapy

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Ebru Sinici

Positive and Transcultural Psychotherapy (PPT) is an integrative approach derived from four main schools: psychodynamic, existential-humanistic, behavioral, and transcultural therapy [1]. According to its founder, PPT, with its own specific intervention methods, as well as together with features of other therapy theories, is a humanistic, psychodynamic, resource-oriented, conflict resolution-focused, complementary, and trans-cultural approach [2]. At the same time, it is a psycho-dynamic approach based on a positive conceptualization of human beings. It focuses on conflict resolution.

The word positive, which is the most important concept of PPT, comes from the Latin word "positum" and means "thing that is posited and factual." Therefore, both positive and negative aspects of an event or situation should be considered together.

PPT has three important principles: the balance model, the principle of consultation, and the principle of hope. According to this concept, a balanced life is possible with a balance within the four dimensions of body, achievement, contact, and meaning/future [3]. Generally, people refer to these four areas: when they face a problem, they feel disappointed, exhausted, or find life insignificant. To be mentally healthy, happy, and peaceful, it is recommended to distribute time, labor, emotional relationships, and spiritual energy in a balanced manner in these four areas [4]. The "principle of consultation" refers to the client's cooperation with the people around him or her in resolving problems. This cooperation is performed by addressing the five stages in each session and in the entire therapy process. The "principle of hope" means that clients are talented individuals who are resolving their problems and believe that the problems can be resolved [5]. The most significant purpose of the therapist is to reveal and foster this belief and hope.

Positive Family Therapy (PFT) is a form of PPT used with families and couples. In the family approach, individuals are handled individually as a part of the system, and the relationship between them is studied. There are several methods for shaping this process. All of these are referred to as family therapy and can be in the following forms: individual/one spouse, couple, nuclear family, extended family, or other systems [6].

To work with an individual/one spouse, at the beginning, although referring to this as family therapy is not meaningful, this approach is necessary for situations in which other people involved in the conflict cannot be included in the treatment sessions. In the context of the belief that a single change in system elements will affect the whole system, the person is given the task of abandoning the patient role and acting as a therapist in

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_23

his/her case. This situation allows behavioral change in the client and thus can have a curative effect on the whole family.

To work with a couple, here, both members of the couple participate in therapy and behavior that they demonstrate in session mirrors how they are affected by each other. In Positive Family Therapy, instead of talking about their conflicts immediately, what holds the couple together and how the conflict has a function in their relations are discussed. Therefore, a common base is formed to cope with the contents of conflict. This situation brings the couple to a new position in which they can look for different ways to resolve their problems.

To work with the nuclear family, parents and children are involved here. Conflicting experiences and their resolution have continued in this family as much as possible. The five-step intervention method is used in PFT to define family rules and their conceptualization. The family uses the system for self-help. This approach helps families clarify and differentiate their positions and make them aware of misunderstandings.

To work with extended family, in addition to nuclear family, other people in close contact may also be involved in the treatment (grandmother, grandfather, aunt, uncle, family friends, etc.). The size of the group can only be limited by reasons due to arrangements. The "family tree" of concepts is often discovered through the patient's memory and experiences and takes its actual form here. Later, these concepts can be discussed in therapy sessions without hurting some members of the family.

To work with Other Systems; here, therapy passes beyond the family limits. External contacts and social institutions (e.g., teachers, directors, physicians, and clinical personnel) participate in therapy. The system to be used in therapy can be modified to cover other subsystems, such as institutions at work, social, and governmental groups. This method complements family therapy and verifies that the family is not an independent or a single entity and is part of the ecological environment. Positive family therapy attempts to adapt the principles of community psychology in the treatment of individuals, couples, and families. As such, by paying attention to social institutions, positive family therapy covers the spectrum ranging from individual therapy to community psychology. Wherever the person starts, the center is still the family. The family is the original group where individuals experience socialization, and it is the place that shapes current relationships and has emotional significance [6].

Transcultural Positive Psychotherapy

The concepts, norms, values, behavioral patterns, interests, and perspectives that are valid in the culture are addressed in the transcultural context. This approach includes the characteristics of a community. It uses community norms and behavior patterns as a guide. One way to objectify these values is to compare the laws and legal norms of the nation. Conflicts, possible solutions, and daily behavior patterns that are typically seen in a culture are perceived differently in another culture. At this point, the transcultural approach seeks to mediate between various views and to find a different form of communication that will help to break down prejudices in discussions about conflicts. These prejudices can be German, American, Iranian, Turkish, peasant, urban, etc. For these reasons, there are examples of transcultural definitions that always allow exceptions and are examples of individual situations. These include the "Prussian" Middle Eastern, who took punctuality, regularity, and accuracy very seriously, and the "Middle Eastern" Prussian, who is highly tolerant of punctuality lapses [6].

Solution-Focused Family Therapy

In solution-focused family therapy, the focus is on possible helpful solutions with less attention given to problems. A basic postulate is that "change is continuous and inevitable." Only a small change is necessary. Changing a small part of the system will also change other parts. The term Domino as a metaphor is illustrative. The

counselor helps the client identify what he/she should do when he/she meets the problem in the future [7].

Psychodynamic/Experiential Family Therapy

This therapy aims to examine the unconscious functioning of the family, i.e., the forces that connect the family or interfere with their functioning. The method is through talk. The way in which the family members talk and begin to talk allows the therapist to work, starting from the transference and countertransference. In addition, the therapist performs a special listening that supports the exchange of relationships and bonding without any instruction or advice. The therapist tries to establish a close bond with every member of the family. While this method is useful in dealing with dysfunctional relationships or after traumatic events, it can also handle problems of parenthood, divorce, and remarriage. In fact, many couples formed by second marriages believe that love and goodwill will solve disputes between children. However, the internalization of the restructuring process always requires an adaptation process. Some supportive techniques that are defined as activation techniques allow us to reveal the dynamics of family relations [8].

Family Tree Therapy

Family tree therapy, based on the idea of intergenerational psychological transmission, uses the family tree as a tool. Traumas, secrets, losses, deaths, and migrations from past generations are transferred to the next generation. It widens the family perspective by including invisible ties between generations to the concept of individual therapy based on the idea that a person's psychological structure is shaped by the family in which she/he was born. Some of the difficulties encountered in life do not belong to the individual; they are passed through the first, second, and even earlier generations. Family tree therapy examines the broad family dynamic, an individual's place in these networks of relationships, and how it is affected. When these knots are united by understanding each other, repetition is prevented by transferring them to future generations. Therapists usually work around a "family tree" drawing that helps them talk. Meaningful childhood and family photos are also used to visualize the family system and help to analyze the family tree [9].

Systemic Family Therapy

Systemic therapies emerged in the 1950s in the United States and include several schools. It addresses individuals, couples, and families. The complaints that force you to consult at the individual level also have a meaning and function in the system in which they appear and hold. To be aware of these issues, the approach is based on the therapeutic alliance. If the family is eager and trusts the therapist, the bond with the therapist makes everyone feel self-conscious. During this one-year process, with an average of one session every two weeks, a therapy system is established that includes not only the client but also the therapist. The complaint of the therapist is questioned in terms of its "cyclicality." The therapist approaches openly so that everyone can finally talk and experience the situation in which the family is blocked in a different way. The basic idea is to give the family a more flexible listening opportunity, which they can reuse among themselves. Another frequently used technique is "reframing," which aims to define a situation in such a way that family members can perceive in different ways and consider new possibilities [10].

Parent Couple Therapy

This supportive therapy addresses couples who face difficulties with being a parent instead of with their marital life. The method was developed as a result of clinical experience with children and is based on evidence. When a mental disorder occurs in a child, individual therapy is absolutely needed. However, in the presence of relational difficulties, working with the child in

parallel with the sessions dedicated to the parent provides more effective results. For example, in the case of different educational needs or when a child or young person is stuck in a conflict of loyalty after divorce, this approach aims to place the child's interests at the center of parental communication [10].

Gottman Method

This approach has been developed to help couples learn about the original skills needed to deepen intimacy and friendship in their relationships. To ensure the efficient management of conflicts, it offers ways to address solvable problems and to establish dialog on deadlocked issues. The Gottman approach symbolizes the secret of happy couples utilizing the concept of the sound relationship house [11].

Holistic Family Therapy (Gestalt Therapy)

Its aim is to investigate the individual's experiences, models, or instances and to integrate and complete all the pieces that are separated. Gestalt is a process used to create or design awareness. Or, it is a process that facilitates the individual to get an idea of the processes such as "what does s/he do, how s/he does it and how it can be changed," and it is a process that facilitates the process to grasp and at the same time learn to accept. It tries to change the family by changing individuals and focusing on the "Now and Here" [12].

Principles of Positive Family Therapy

- *Self-help:* PFT is a self-help method used to help resolve family conflict. For instance, rather than focusing on a child's problem or parental conflict, it teaches the whole family to mobilize their healing capacity that is already present. The focus point of the therapist is the family. While addressing the family, the therapy addresses the values, impressions, social norms, and cultural characteristics that already exist in it. The therapy method applied to families or couples is taught during the therapy. Therefore, the aim is for a person to be able to use this method for themselves or their family after the therapy ends.
- *Transcultural Dimension:* The family or couple should be handled together with their cultural environment because it is not independent of that cultural environment. PFTs argue that it is possible to use appropriate methods and meet their needs fully only after understanding the cultural characteristics of the family.
- *Overview of the Content:* This section seeks to answer the following questions: What are the points where people are common and differ? It uses the Differentiation Analysis Inventory (DAI) and stories to do this.
- *Meta-theoretical Dimension:* PFT provides a holistic approach by incorporating insights from many theories. This approach provides a framework in which different therapeutic techniques can be applied and integrated.
- *In relation to family bonds,* PFT is a special form of therapeutic thought. Although the family is at the center, therapy is not limited to the family; family members are regarded as individuals, and social factors influence treatment.
- *With respect to the uniqueness of humans*, PFTs attempt to understand the psychological and psychosomatic reactions of people to conflicts. It examines how, and with what content, these responses emerge in terms of personal sensitivity. At the same time, it also considers understanding the rules governing everyday life among the factors of reactions, culture, traditional values, and belief systems as significant elements of the therapy process [5, 16].

Actual Capacities and Differentiation Analytic Inventory

The Differentiation Analytic Inventory (DAI), one of the basic instruments of Positive Family Therapy, reflects the rules and components

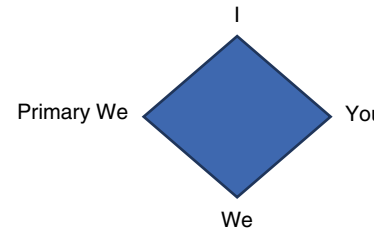
behind the conflicts that manifest themselves in a series of repetitive behaviors or norms behind the symptoms of people's disorders. Within the scope of this inventory, norms of behavior such as punctuality, openness, kindness, cleanliness, loyalty, and justice are called actual capacities [6]. The DAI is an inventory in which concepts related to individual, cultural, and family conflicts are gathered, and it is a guiding inventory in the determination of actual capacities; the short form of the DAI is included as an appendix to the Chapter on the First Interview.

Actual capacities are psychosocial norms that come from family, social values, and belief systems. They constitute the rules that define the relationships between family members. The obvious reflections of actual capacities are hidden in the behavior patterns that seem to be very unimportant [13]. For example, for the boss looking at the clock continuously, punctuality is an actual capacity hidden in the background. At this point, the differentiation analysis begins with a detailed assessment of actual capacities, which include both development and conflict potential. Information such as "When the person is angry, with what she/he is contacting and the reasons for explaining the situation, etc.," are gathered. With this analysis, how sensitive a person is to conflicts is defined, and the real reasons, concepts, and special contents behind emotional conflicts are understood.

By applying the DAI to each family member, we can obtain information about the entire family. In the table illustrating this situation, the person's own data are shown in the "I" column, assessments of the person's wife or other family member are depicted in the "Spouse/Partner/Conflicted person" column, and comments about this capacity are indicated in the "Spontaneous Answers" column. The person can bring these assessments together about her/himself and the other person with whom he/she is experiencing conflict and can obtain a general overview. The assessments here are subjective assessments, not decisive judgments. The individual standards of family members are compared with each other. It can be easily understood which family member's capacity is more developed or which of them need to be improved. Therefore, these conflict areas and their contents can easily be identified.

Model Dimension

According to Positive and Transcultural Psychotherapy, actual capacities develop according to relationships with parents or primary caregivers and childhood life. Peseschkian, with the concept of the Model Dimension, defined the development of actual capacities and how to understand them. The Model Dimension defines the basic relationships in which each person is involved in one way or another. These relationships are "I, You, We, and Primary We." Positive Family Therapy also demonstrates how the actual capacity in humans develops according to the person, time, and environmental conditions by using the model dimension.



The Model Dimension concerns concepts that are valid within a family. These concepts can be impulses that produce behaviors, norms, and habits that lead to these impulses. These concepts have special importance in social life in PFT, and they are cognitive and emotional structures that reflect relationships with the environment. In these concepts, there are several expectations, such as a critical and pessimistic approach to the person in a relationship or the exact opposite, the desire to establish a close relationship. The model dimension is defined as revealing the concepts of the family in which the person grows and the experiences of the person about these concepts. To understand conflict, first, it is necessary to understand its infrastructure and contained concepts. The Model dimension is helpful for understanding why people prefer or reject some relationships. It sheds light on the relationships

and values of people in their families. A person can combine his/her relationships with other people, and the models symbolize these experiences [14].

The "I dimension"is developed by taking a person's relationships with parents and siblings as a model; the "You dimension" is developed by experiencing the relationship between one's parents as a child; the "We dimension" is developed by the example of one's parents' relations with other people; and the "Primary We Dimension" is developed by experiencing one's parents' meaning in life, attitudes toward religion and relations with spirituality. The basis of the behaviors and the models taken can help the client be informed about their emotional relationships and can help to understand the source of their problem and provide him/her with data to evaluate the situation from a different point of view.

Stages of Interaction

Humans follow certain stages of interaction in each relationship they establish and will establish. Each stage has specific characteristics, and these characteristics help to understand the needs and expectations in relation to and dynamics of the relationship and help in understanding and solving the conflict. Therefore, people go through three stages in marriage and in couple relationships: attachment, differentiation, and detachment. In each close relationship and couple relationship, these transitions are seen continuously. In PPT, we speak about the "three stages of interaction" (Peseschkian [3]):

The Attachment Stage: Every human being requires the togetherness and intimacy of others throughout his/her life from birth. This need greatly explains the person's search for a marriage partner, his/her desire to be with other people and his/her commitment to the family community. It can be understood from the sentence "I want to be with my spouse more often, I need her/him." that the person is in this stage during a marital relationship.

The Differentiation Stage is defined by the acquisition of socially desired behavior. This situation occurs in the differentiation of the person's ability to recognize and learn and in the formation of the secondary capacities that enable him/her to control nature and express her/himself socially. Trying to attract someone, giving advice, and trying to control someone's attitudes and behaviors are the obvious behaviors of this stage. It can be understood from the sentence "Do I advise my partner/spouse? What is my response when my expectations are not met?" that the person is in this stage during a marital relationship.

The Detachment Stage refers to the period when the person is separated from the person, she/he is close and when the person is looking for things, he/she needs freely and when the person starts to carry his/her own responsibilities. This stage provides an opportunity to establish relationships with other people, and it means expanding one's qualifications, trying new decisions, or reconsidering one's old values. This is the characteristic of maturity and a mature personality. It can be understood from the sentence "Do I expect my partner to be independent? Do I prefer to give him/her responsibility anymore? Do I think that she/he has the right to take care of him/herself?" that the person is in this stage during a marital relationship [6].

Most people alternate between attachment and detachment. They want to be independent, and they see that they cannot manage while they are independent. Or, they want love and the interest of their spouse, and they avoid it because of their ambition of freedom. We call these kinds of relations two different attachments. These kinds of people are under the influence of sudden powers and new possibilities outside them. They are seen as a type of people who are not certain what

to do with their environment and themselves.“ I both want and do not want.”

Clinical Examples

These three stages of interaction can be seen in every kind of interpersonal relationship. Each person has other needs and expectations and might be in another of these stages.

(a) For instance, a 3- to 4-year-old girl wanted to play with her father (stage of attachment). However, the father retracts himself, stating he has no time (stage of differentiation).
(b) One partner wanted commitment, and the other thought that he should provide information and warnings.
(c) A working woman waits to show compassion to her husband in the evening. However, the husband asked, “Kitchen is untidy, toys are everywhere. Why did I marry?”
(d) One partner wanted commitment. However, the other gives a different commitment than she expects. An 18-year-old young person is invited to a party organized by the family. She/he is not comfortable among the adult people. However, as she/he tries to feel confidence through her/his closeness to her mother, the mother smothers him/her with kisses. In addition, the child feels very ashamed.
(e) While one partner needs information, warning, and verbal instructions, the other expects him/her to be independent, to make his/her own decisions and to support his/her spouse. For example, a young boy asks his mother for advice about his young girlfriend, whom he loves but cannot trust. The mother may reject him by saying “You have never asked me before, you managed by yourself. Why do not you ask your father?”
(f) One partner asks for information but does not get it from the person she/he asks or gets in a way that she/he does not desire. A housewife says, “The washing machine broke down last week. I wanted to hear from my husband what I should do. However, he gave me a lecture about how to use the machine more carefully.”

Here, one of the partners needs information, warning, and help in decision making (stage of differentiation). However, others want to reach a stage of commitment based on care and attention.

(g) A working person wants to obtain an idea about training in his/her profession. If her/his visitor is a mother, she may say “It is obvious that you have much to do and you look bad. Come stay with me. I will let you rest.”
(h) One of the partners wants to prove him/herself. The other one is not aware of this desired or obtained independence. In contrast, she/he wanted to control his/her couple by providing ideas and suggestions. The mother of a newly married woman will visit. The mother says, “I am glad I came. Everywhere is dust. Now, your mother will show you how to clean.”

Here, one of the partners wanted independence and separation, while the other took this as a dependence need.

(i) A young girl is admitted to a college in another city. The father may reject this by saying “You cannot go thus far away. Something bad can happen. There is no need to go to university. Stay in your home. “ [15].

These types of interactions may provide an orientation for analyzing conflicts between one’s partner or inside of an individual. Person with the help of these may have a good understanding of tentative problems and can also consider his/her own attitude.

Five Stages of Positive and Transcultural Psychotherapy (Principle of Consultation)

The core of the PPT is a 5-stage process that serves as a guide for shaping the therapy. While applying these five stages of therapy, PPT also benefits from other therapeutic approaches.

1. *Observation/Distancing Stage:* The basic objective of this stage is to analyze the situation. First, the therapist listens in an objective manner without commenting and tries to understand what he/she needs with what the client brought as a problem. Thus, the therapist tries to examine all sources of information both situational and past about the symptoms and the accompanying behaviors. Families and couples have an opportunity to look at the current situation by going one step back. PFTs use several tools to maintain distance from the problem. With stories, proverbs, and how other cultures experience these tools are used to reveal new points of view.
2. *Inventory Stage:* In this stage, by using a balance model, the information about what they experienced in the last 5 years is gathered in the four life dimensions. The therapist tries to determine how they solve the problems they face and which dimension of the balance model they use more. In addition, at which level they use actual, and capacities is examined. DAI is used at this stage.
3. *Situational Encouragement:* Situational encouragement has central significance for PPTs. Since families focus only on their problems, they cannot recognize their positive aspects and capacities. At this stage, the family and the couple are encouraged about the positive aspects of the situation from a realistic point of view. The therapist tries to help them in finding new alternative ideas.
4. *Verbalization:* After the creation of the necessary therapeutic environment in the previous stages, basic conflict began to be studied in this stage. The participants were asked about their thoughts about apparent problems and deeper problems. The therapist tries to help the family solve their conflicts by using the balance model and the model dimensions. They learn about each other's ideas and expectations. The main objective in this stage is to emphasize what the family and the couple can do to achieve balance in the four dimensions of life.
5. *Broadening of goals:* In this stage, after individuals reach the application goals of family and couple, they are focused on how they want to live in the future (in the short, medium, or long term). In this stage, the main goal of the psychotherapy relationship is to provide information to the family as self-help. Therefore, people will be equipped with a self-help method to accompany them during their life after stopping therapy. In this stage, the family learns to reconsider the goals pursued before neurotic restrictions restart. While doing so, the family takes the four dimensions of life in the balance model and plans how to maintain balance in their life and examines which methods to address them when new conflicts occur [14].

In summary, positive family therapy, due to its structured techniques and as a transcultural, psychodynamic, and system-oriented method, has proven to be effective in 8–12 sessions, and the goal is to assist patients in using their own resources.

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Positive Pedagogy and Counseling

24

Stefanka Tomcheva, Snezhanka Dimitrova, and Ivanka Boncheva

An elephant had been brought into a dark room at night for an exhibition. The people streamed by in throngs. Since it was dark, the visitors couldn't see the elephant; so they tried to get an idea of his body by touching him. Because he was large, each visitor could only grab a part of the animal and describe it according to what he had felt. One of the visitors who had gotten hold of a leg, explained that the elephant was like a mighty pillar; a second one, who touched a tusk, described the elephant as a pointy object; a third, who grabbed the creature's ear, claimed he was not different from a fan; the fourth, who ran his hand over the elephant's back, stated that the elephant was as flat as a couch.

(After Mowlana, cited by Peseschkian [6], p. 82)

Pedagogy is the science of education and upbringing of adolescents. Pedagogical activity is associated with the concepts of wisdom and experience, and in it are found (it is necessary to be present) actual abilities such as: patience, attention, model/pattern, discipline, responsibility, achievement, and beneficence.

Education and training are a necessary part of human development. Education and training prepare children, adolescents, and young adults to know and be able; education prepares them for adult life—the necessary knowledge, skills and habits, understanding and acceptance of the written and unwritten rules in society and communication (interaction) with others.

As a practical activity, pedagogy is in the hands of teachers in different types of schools. A teacher in modern times gives priority mainly to education, to giving knowledge in a certain scientific field (subject), including the science of ethics. Therefore, they prioritize themselves as teachers of a certain subject. Education, the practical "teaching" of moral rules of communication, begins right from the primary school in the immediate contact between the child and the teacher. The child hears verbal demands on how to behave in the group; a moral evaluation of the success of his behavior. They learn the science of ethics in the course of actual communication with teachers and classmates. The teacher's behavior in this type of communication fully covers the social role of "mother" or "father" learned by them over the years and based on their own personal skills.

In the course of their professional growth, teachers, regardless of their personal characteristics, predominantly develop and use their secondary qualities—order, discipline, punctuality, consistency, time frames, success, achievement, and evaluation. This is in keeping with the aims of the institution of "school" and with the original social task of imparting, as appropriate to the age of the child/adolescent, the minimum knowledge required in various scientific fields.

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_24

There are situations in which the teacher is challenged upon facing the needs of the children/adolescents to know themselves, to feel themselves, to use their own resources for self-affirmation, self-improvement, self-acceptance, and self-development. The teacher is expected to be a role model and to point out ways of learning and integrating personal qualities such as humanism, tolerance of the views, morals and habits of others, how to approach complex social situations creatively, what the organizational, communicative and emotional-volitional skills expected of a young person look like in practice. This poses the challenge for teachers to "meet" first and foremost with themselves, and to unfold their knowledge of their own needs, desires and expectations—to know the meaningful aspects of their experience (primary capacities) and those of their behavior—beliefs, attitudes and values (secondary capacities), and the importance of the cognitive-emotional relationship between them—the concepts that guide them and that leave their invisible mark in relationships [9].

As a rule, the teacher enters the social role of "educator" on the basis of their own personality characteristics. Predominantly at such times the teacher is in the role of the "father" who imposes the norms and has requirements that are respected. He becomes strict, demanding, consistent in upholding social norms, socially just.

In supervisions and Balint groups conducted by school psychologists with psychotherapeutic competence (trained by trainers at the School of Positive Psychotherapy), teachers share conflict experiences that can be summarized in the following main themes:

- Attitudes to their professional identity and role.
- Insufficient effectiveness of teachers' own behavior to deal with everyday school situations without conflict.
- A clash between their own perceptions and expectations (concepts, models) and the reality of the relationship.
- Difficulties in reflecting own feelings regarding topics such as power/strength, anger, control/self-control.
- Bearing emotionally charged content in the process of communication and lack of emotional communication skills.
- Developing and strengthening skills of connectedness, support, differentiation, and separation.
- Building a pedagogical identity from the position of one's own resources and constraints (work ecology and psychological hygiene).

The help of the psychologist—a positive psychotherapist—assists in the integration of their professional training and their inner needs, in the recognition of students' needs for self-knowledge and development, and in the development of skills to manage conflict. Each of these areas will be described below.

Developing Skills to Integrate External Professional Training and Preparation (Forms, Models of Professional Reflection) and the Inner Need to Realize the Personal Meaning of Professional Self-Development

An effective teacher is like a good parent. Teachers who have undergone psychological training with psychotherapists already have a deep psychological knowledge of how to harness the power of the "***balance model***". After systematic training in self-awareness and a series of supervision sessions, they now have the skills to recognize the mechanisms that ensure professional and personal self-development:

- Self-awareness—understanding, knowing, accepting oneself.
- Self-determination—a sense of one's own limits.
- Self-actualization—the notion and understanding of one's own resources and potential.
- Self-activity—the teacher as a subject in the field of his professional activity and in which he discovers and develops his own self.

- Self-regulation—the management of various aspects of personality and their integration.
- Self-development—the purposeful development of certain (significant) abilities, aspects, and qualities.
- Self-identification—identifying oneself and a position or role.
- Self-esteem—a balance in the relationship between the "Real Self" and the "Ideal Self".
- Self-actualization—acceptance of oneself, personal values and the values of others, ability to develop oneself, getting satisfaction from this process [9].

Self-monitoring with respect to certain psychological contents is particularly significant for professionals working in the human-to-human professions. In this sense, in the profession of the teacher as a role-model, self-knowledge is on the one hand an opportunity to explore the "power of one's own personality" and to allow for "patterned learning" (McConnaughy 1987) [3], on the other hand a basis for building professional identity and successful prevention of burnout syndrome. According to I. Boncheva "self-knowledge is a moment of insight (enlightenment) about the existence of certain, appropriate psychological preconditions, thanks to which the learner, self-defines himself in relation to his professional role" ([1], p. 177).

N. Peseschkian teaches teachers to recognize the psychological content, localization, and areas of conflict processing:

- In the realm of the rational (cognitive)—on the one hand, professional knowledge and beliefs, awareness of the "professional self" and on the other hand, differentiation of one's own concepts, needs, and expectations, awareness of the self in the sense of inner harmony and psychological maturity;
- Emotional domain—emotional- evaluative attitude toward professional expectations, socially significant for society, others, and expectations toward oneself—emotionally significant for the individual;
- Behavioral aspect—style of relationships in the professional environment and boundaries, and aspects of behavior in the context of the functioning of the person as a whole.

Having seen this, teachers are able to find more appropriate ways of responding to situations that upset their internal mental balance by bringing a different experience and/or a new reading of their old approach.

Teachers' reports showed tendencies for certain strategies to be repeated.

In the ***Body area*** (Identity—"Who am I?"; "What am I?")

According to L. B. Schneider [8], professional identity implies conscious attention to the question "Who am I?", which is present in one's inner world throughout life and whose answer largely determines professional activity. Teachers feel squeezed in their professional identity as a result of intense changes in reality, leading to new and new demands on the training and improvement of professionals. From the point of view of positive and transcultural psychotherapy, professional identity carries the emotional meaning of the primary capacity of "connectedness/unity" and in this sense is necessary for the teacher to align himself with others/the profession, to feel himself part of a whole and thanks to this to see and perceive his uniqueness and uniqueness. Professional identity implies a functional and existential fit between a person and a profession, which includes an understanding of one's own profession, an acceptance of oneself in the profession, an ability to perform professional functions well, a sense of competence, self-efficacy, and personal influence and significance [8]. The teacher brings in key moments from his/her life and work experiences, shares personal qualities and actual abilities to deal with different activities and to interact with different people. Within the process of self-discovery, is able to observe, reflect and respond to the individual style of others (colleagues and students)—skills of relating and differentiating, need to bring self and own experience. For the self, the teacher learns a new manifestation of role behavior, makes sense of character traits, new interactive abilities (activity, reactivity, passivity) and coping strategies; recognizes and inventories the contents of his/her

own mental balance in the Body domain: health, health problems (symptoms); pleasure in contact; satisfaction with activity and roles; the unconscious and its manifestations.

The teacher's need for professional satisfaction, development, and growth introduces the process of self-discovery into the ***Activity area*** (Motivation, Achievement, Deeds). How did the need for teacher education arise? How, and under what conditions are actual abilities developed? What is motivation, and what provokes the upgrading of personal professional skills? Motivation is the drive to action, the ability of a person to satisfy his needs through some activity. Motivation is a dynamic process that controls human behavior and determines its organization, direction, stability, and activity. The discussion on professional satisfaction inevitably leads to the topic of self-improvement, which at its core is a person's need to surpass his or her current self, to achieve higher results, to improve skills, and to develop personal qualities that he or she identifies as significant to him or herself. It is a process that is underpinned by one's capacities to learn and train (secondary capacities): *persistence, diligence, consistency, discipline, responsibility, achievement, and success.* Constantly working on oneself in order to change, reach the ideal of the Self, and as an end result, personal growth. In the course of self-discovery, the teacher clarifies their own picture of themselves, their strengths and weaknesses—important elements on which self-acceptance depends. Whether one accepts oneself or not, as a rule, determines the choice of strategies for self-improvement. Hence the conclusion for the teacher that in order to feel professionally satisfied, motivated for achievement and success, it is necessary, on the basis of knowledge about oneself (personality traits, needs, qualities, skills, abilities) to create for oneself a self-development program based on order and sequence, timing, conditions, outcome predictions, methods, and techniques of self-education and self-improvement.

In the ***Contact/Relationship area,*** the process is unfolded on two levels: contact with self and contact with the environment. In terms of contact with the self, topics related to self-esteem, self-respect, and self-worth are significant for the teacher. Issues of interest to the teacher relate to belief and confidence in self; own strengths and weaknesses; patience and tolerance of other points of view; flexibility and adaptability, experiences of comments about professional role and criticism of performance of professional duties; the need to discuss openly their personal and professional problems or have to deal with them themselves; the ability to hear negative feedback from the other and what is the "cost" of trying to cope with experiences. When discussing the contact with the environment, the most frequently used interaction models were presented with a focus on the contents: proximity-distance; abilities to connect with colleagues/students, as well as similarities and differences between using the same abilities with relatives, friends, partners; the importance of information that reflects the "social mirror"; pleasure and satisfaction from the contact. The teacher discovered his ability to observe group dynamics—formation, struggle, norming, transformation and the maturity of the group (class, pedagogical collective) in terms of interactive abilities—asserting one's own stake in group work—responsibility and ambition; reactions to demands, norms, rules; skills in dealing with conflict situations, attracting like-minded people and confronting the other; seeking and giving help; trust—ability to rely on each other; differentiation—competence and independence in terms of decisions made and goals and objectives achieved.

Fantasy/Future Area At a conceptual level, the observation of the behavior of individual members of groups of students, colleagues, provokes a discussion about the capacity for self-regulation—the ability in different situations to bring different roles and the need for flexibility in dynamic adaptation to the other and others. Concepts underlying one or another behavior were commented on, forms of resistance and psychological defense mechanisms used were also visible. The process of self-discovery touched on themes of strong emotional significance for the teacher—the meaning of human existence; life as a struggle to assert oneself; the ability to carry the burden of life's dynamics and

the reasons for undertaking change. Commenting on his personal identity, the teacher spontaneously moves on to a discussion of significant individual and social values, definitions of the same, and linguistic pictures of their content: truth—own and other, obligation or conscience; crisis as chance—a way to the missing; silence—punishment or value, a challenge to the other; emotionality—breakdown or benefit; boundlessness and unreservedness—the meaning of limits. The content explored in the Fantasy/Future domain led the teacher to reflect on a philosophy of life—his own and that of the social groups in which he functioned, to arrive at the idea of a meaningful, measured relationship to his students, colleagues, and school leadership. This allowed the teacher to bring about a change in the content of his interactive behavior, from a need to know more about others and to receive more from others, to a willingness to share position, personal experiences and plans for the future. As a result, the teacher demonstrated the ability to use the concepts of "Positum" and "Unity in Diversity".

Having mastered ***the concept of Positum***, teachers become good role models for their students, guardians of their own and their students' mental health. This happens most successfully when the teacher demonstrates respect for each participant's available psychological capacities as a coping resource. In the course of working with the psychologist-positive psychotherapist, the teacher learns the skill of using the ***transcultural approach*** to ***positive reinterpretation***—the skill of discovering the function of the student's behavioral act.

Recognition of the Child's/Adolescent's Needs for Self-Knowledge and Development

As a mental process, self-knowledge is responsible for the integrity and unity in the development of the individual. It is a lifelong process that occurs on three levels:

- Biological—knowledge of oneself and one's own body.
- Social—learning, skills, and understanding norms of social functioning.
- Personal—the right to choose, make decisions, and determine one's personal life path and the meaning of life.

From the perspective of positive and transcultural psychotherapy, self-knowledge has meaningful characteristics visible through the four domains of human functioning:

- Health—"Somatic Self-Feeling" (Body area) ([5], p. 96).
- Personal potential—concepts of success, achievement, ambition, fulfillment (Activity area).
- Abilities to relate to the outside world, others, and self (Contacts domain).
- Inner process of spiritual and moral self-development (Fantasy/Future area).

Content-wise, the four spheres have a unique character, role, and meaning, for each person [7]. They are individual and dynamic and change over time and through different stages of individual development. Self-knowledge seeks to achieve one condition—harmony (inner peace and psychological maturity).

Self-knowledge, self-understanding, self-development are an important condition for effective learning and education, an integral part of the overall pedagogical process. Self-knowledge is connected with reflection, identification, with the discovery of one's own "I" and its interweaving in the system of social relations, the formation of basic character traits and properties of the student's personality, his spiritual world, communicative abilities, and cognitive activity. The teacher who guides students in the process of self-discovery needs to be a highly qualified specialist with a high general and pedagogical culture, to master certain techniques of self-discovery and, above all, to have undergone and experienced this process himself.

The idea of any training and education is to be developmental and non-violent, to integrate into

the content of the educational work system the value-meaning context of the student's self-development, to involve students in various kinds of individual projects and research activities, activating the inner resources for self-development. In this sense stands the classical postulation of Vygotsky's [10] zone of actual and immediate personality development. The zone of actual development is everything that the student can do independently, without adult help. The zone of proximal development is what the student does today with the help of adults: parents, teachers, educators, and tomorrow will do independently.

Teachers going through their own self-discovery were given the opportunity to learn about and use the 5-Step Intervention Model as a self-help tool and to assist their students in the self-discovery process:

Current capabilities presented in 5 steps [2].

1. *Observation: what is happening to the student, what is changing; what attracts their attention, how they seek attention, what is interesting to them, what is not; what they like, what scares them, etc.*
2. *Stocktake (Inventory)—what are they doing at this stage, what is making it difficult.*
3. *Encouragement (Situational Encouragement)—drawing the student's attention to anything they are doing well (specifically named, even if it is something minimal).*
4. *Explanation (Verbalization)—naming the specific activities the student is doing and his/her skills and abilities (accuracy, order, observation, attention, patience, discipline, etc.).*
5. *New Horizons (Expanding Goals)—the student is willing to learn new skills—they are shown something different and encouraged to try on their own. The first attempts need to be with the help/support of the teacher/educator, then they need to try on their own.*

Due to the particularities of the respective age crises, children/adolescents respond predominantly with their primary abilities. They are impulsive, spontaneous, uninhibited, immediate, highly emotionally charged, and with a need to assert themselves and to resist demands and expectations on them. In a series of supervision sessions, teachers brought in cases related to motivation for learning and self-development and the abilities woven into them—*persistence, diligence, consistency, discipline, responsibility, achievement, and success*—secondary abilities. With the help of the positive psychotherapists leading the supervisions, they were able to see a way in which to link what was expected of the students (secondary abilities) and their needs (primary abilities).

Expectations for Students

- *Stubbornness and consistency*—the abilities of a person, helping them to remain faithful to the idea, the task, the goal they are pursuing, despite setbacks and obstacles. Stubbornness and consistency are self-discipline in action. Achieving a goal may require repeated attempts, and with each attempt one definitely moves toward it, gaining experience and building strength of character along the way. Perseverance and consistency mean determination to finish what one has started.

Relationship to Need

- *Persistence* is a personality trait that represents a person's ability to achieve specific near-term goals despite obstacles and setbacks. It is ***fidelity*** to the goal. It is important to help the student not confuse persistence with stubbornness. Stubbornness is rigidity and does not allow for variability in solving certain problems. Stubbornness forces one to tread in the same place—"There is no worse craft than stubbornness!" (folk wisdom).
- *Consistency* is an ability that allows a person to pursue long-range and long-term goals. It is ***time and patience*** in relation to goals.

Expectations for Students

- *Diligence*—it is an inner state and positive quality of a person, expressed in the manifestation of care.

Relationship to Needs

- *Effort* is ***attention*** to what is being done. Approaching something with diligence means focus, concentration, absorption so that everything is done properly.

Expectations for Students

- *Discipline* is strict adherence to the rules and what has been agreed. It implies penalties in case of non-compliance. Reasonably organized systematic enforcement based on strict compliance with the rules in place.

Relationship to Needs

- *Discipline* is ***self-organization***, control over one's own actions. How to learn? Forcibly—demands are imposed, orders are given, deviation from the "correct" (imposed) path is not tolerated for effectiveness. Softly—the opportunity is sought to present the requirements in such a way that the object of discipline becomes interested and wants to do what is expected (not to be confused with obedience). Tactical—creating the conditions so that self-organization in the right direction happens naturally.

Expectations for Students

- *Responsibility*—an actual ability involving the attitude to approach any matter thoughtfully, carefully and, if a decision has already been made, to see it through to the end. Responsibility fosters courage to suffer the consequences. The inner basis of a person's responsibility is his or her authorial stance, the belief that the events that happen to him or her depend primarily on him or herself and his or her actions rather than on external circumstances.

Relationship to Needs

- *Responsibility* is a ***model/pattern***. It is a burden and most people tend to avoid responsibility. Usually people tend to be responsible only for what is in their circle of interests. The more a person is strong and developed, the more they can, (are willing to) and want to be responsible. As a rule, an adult is responsible (should be responsible) for his words and deeds, for his thoughts, for his feelings, and through what he does to set an example to the one he teaches (the child/adolescent). And the child/adolescent willingly learns responsibility because it is also a passport to the adult world, a "ticket" to greater rights and opportunities.

An area of discussion for teachers in supervision, and especially in the process of self-discovery, was behavior and the difference between behavior as a constant set of responses (motor and verbal) to external or internal (in their experience) stimuli and professional behavior as stereotyped behavior with its imprint specific to a given professional environment. Different models were discussed [1], whose choice and advocacy depends not only on the personal preferences of the teacher, but also on the students' issues:

- *Model: "Surgeon's Behavior"*—works in the underbelly of the negative, names it as negative, forbids it, brings logic as to why such behavior is not supported, based on the understanding that when the bad is "cut out" of the student, they will find the inner resources to work out the good themselves. Such behavior works for highly intelligent children raised and educated in a healthy family environment. Naming the problem and reminding the rules is sufficient help for the student to correct his behavior.
- *Model: "The therapist's behavior"*—this variant of behavior brings the specific "medicine" into the course of dealing with the problem. Applied to the teacher's behavior, this means not only naming the problem, but mainly communicating and monitoring for implementation the ways in which this problem will be addressed in progress. The teacher upholds the understanding that behavior will not be corrected in the moment, but as a result of systematic pedagogical intervention over time.
- *Model: "The behavior of a sculpture"*—works "piecemeal". Based on daily occurrences, the negative is named and a behavioral option for correction is immediately sug-

gested. Such should be the behavior of the primary teacher.

- *Model: "The Gardener's Behavior"*—starts from the understanding that the student's behavior (especially at primary level) is a newly germinated seed. It has its powers of growth, but it does not know how to care for its place under the sky, its patch of sun, its sip of water. Only the skillful gardener knows where to sow the tender plant, how to feed it, how to weed the unnecessary around it.
- *Model: "In the field of strong qualities and abilities"*—it is often enough to support, respect and allow the good and successful in the other to exist. Even the mischievous, disobedient, undisciplined student has his positive and good abilities and behavior. Timely discovered and given a field for expression, they will gradually unfold to the extent of "pushing out" or modifying the negative ones themselves.

For such professional behavior it is necessary that the teacher refrain themselves from commenting on the bad in the student, naming and directing to the real manifestation of the good only. We return again to the classical staging of Vygotsky [10], which points to working in the field of proximal development. Every successful teacher probably has their preference for one of the models. Some will prefer a dynamic mix of the models. How the teacher proceeds depends on: (a) the concepts he or she draws on; (b) the particular situation; c) the goal he or she wants to achieve.

Developing Skills for Dealing with Conflict Experiences

To train new skills means to learn from your own bitter experience and through understanding try to change it. Peseschkian's (1987) [3] "*How I did vs How I should have done"*. Communicating with the psychologists-positive psychotherapists, participating in self-awareness training workshops, and receiving self-help support encourages and stimulates teachers to reflect and adopt new practical approaches. When a person chooses their path for self-knowledge, personal development and self-realization, they compare their desires: "want"—interests, desires, aspirations; with the possibilities "can"—self-knowledge of one's own capabilities, self-attitude, self-confidence and the need "should"—self-organization, self-regulation. The content of these categories determines the meaning of a person's life. The condition for choosing an effective strategy of self-realization is the balance that a person brings between "want", "can",

Table 24.1 Changes for teachers after self-discovery

How I acted/acted before self-knowledge	How can I act now
With irritation and desire for sanctions when discipline does not match my expectations.	I name the problem and give options for dealing with it.
The lack of precision drove me crazy, I sought an explanation from the other and openly chastised him.	I see the individual parameters of one's ability to actively plan one's time and one's behavior, or to adjust to the given time and to bring one's expectations to others for punctuality.
I took obedience as an indicator of good upbringing.	I learned to distinguish between obedience and submission.
"Let there be order!" and "Let me put things in order!" was my main position. Lack of order meant to me a waste of time.	I see and understand that each person has their own understanding of order and right to organize events and space themselves.
I demanded of myself and expected of others to be kind, friendly, polite, because that's the accepted way, and because that's the point of education.	I've learned to differentiate between situations where being "just polite" means a strong desire to be liked at all costs and those where saying "no" means setting "healthy" boundaries.
They were convinced that things should be named outright, even if that seemed rude. Honesty educates.	I realized that being honest and telling the truth does not mean aggressively defending one's own, but it is a position of self-assertion and self-assertion.
I used to get upset and angry when I observed injustice around me.	I felt how subjective justice can be and how the past (then and there) influences the experience of the "here and now" and what power it has in relation to the future.

and "should". Table 24.1 shows examples from our practice that teachers recognized and shared as the change they achieved after undergoing self-discovery in the Positive Psychotherapy method.

In the beginning we brought in, not incidentally, a parable brought to us by Nossrat Peseschkian [4]. With it we wanted to show another essential tool of positive psychotherapy—***working with parables, stories, language pictures.*** This is an indirect way of entering into the conflict situation and helping to resolve it. Teachers have rediscovered in the use of this toolkit a pedagogical ability of their own—to illustrate what is being experienced. What was new for them was that they could use this same ability in dealing with emotionally intense conflict situations, here and now.

Conclusion

With the new educational skills acquired through the work with the psychologist-positive psychotherapist, the modern teacher acquires a new image, restoring the ancient requirement for the work of the educator—to be not only a presenter of knowledge, but also a person who educates, accompanies children and adolescents in their psychological and social growth, their path to self-understanding, self-knowledge, and self-awareness.

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Positive Group Psychotherapy

25

Ewa Dobiała

Introduction

In positive psychotherapy (PPT after Peseschkian), the group psychotherapy dynamics is reflected on by both the individuals forming the group and by the group as a whole. This multiple-level understanding has been developed and described for several decades by theorists and practitioners representing different approaches: Bion 1961; Berne 1966; Whitaker and Lieberman 1964; Yalom 1970 [6].

Historically, the roots of human perception as both a separate entity and as part of the group can be traced back as far as Aristotelianism. However, the first scientific study of the effect of a crowd on an individual, meeting the contemporary criteria of psychological research, dates back to 1885 and was published by Gustav LeBon, French physician and social psychologist, who first studied crowd psychology and coined the term "group mind." Social psychology pioneers focused mainly on large group (crowd, mob) mechanisms. Sigmund Freud carried out similar observations in groups such as armies or nations [12]. The precursor of current understanding and application of group therapy was Kurt Lewin [9], the American psychologist born in 1890 in Greater Poland (the area of Poland that was the Prussian Partition), and educated in Poznan and Berlin.

Positive psychotherapy—at times referred to as positive transcultural psychotherapy—is a resource-oriented, transcultural, inherently and systematically integrative method, combining psychodynamic understanding with systemic reflection and humanist views. The founder of the approach [10] has developed a concept of basic and secondary capacities that enable understanding and describing of intra- and interpersonal conflict dynamics within the same conceptual framework [4]. Such a perception of the dynamics and origin of an individual's conflicts makes it possible to work both on interpersonal conflicts in individual therapy and intrapersonal conflicts in group therapy (see Fig. 22.4).

The role of a group therapist evolves with subsequent stages of a group process and with different processes taking place in each of the dimensions reflected upon. In the initial stages of the therapy, a therapist acts as a tutor of the phenomenon, helping the participants to set the limits and verbalize the goals. During the subsequent stages, a therapist becomes a source of tools and methods, in addition to a container for emotions, transference, and countertransference experienced in an interpersonal and group dimension. The therapist also moderates the setting and atmosphere in the group in addition to the therapeutic relationship. The therapist's role is determined not only by the stage of therapy but also by

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_25

the level of the occurring phenomena. In group therapy, the intrapersonal dynamics overlaps with the interpersonal, group and systemic dynamics and with the group process to take place. All these dimensions affect one another, potentializing the processes occurring at a given level and are filled with content contributed by the group members.

The group develops conscious and unconscious dynamic interaction patterns that evolve with time and constitute the unique experience of togetherness. These blending processes constitute the social environment and determine how group members experience themselves and one another, affecting their behavior in this social environment. As part of psychotherapeutic intervention, the group is guided to reflect on a given dimension. Well-balanced interventions, tailored to the needs of the group, enable smooth transition to subsequent stages of group development. The achievable goals are somehow determined by the within-group diversity and by ensuring its optimal level.

Definition and Key Concepts

Aspects (Facets/Dimensions) of Group Psychotherapy According to Positive Psychotherapy

Intrapersonal Aspect

Positive psychotherapy has developed an extensive conceptual apparatus to characterize intrapersonal dynamics (conflict types: actual conflict, basic conflict, inner conflict, and key conflict (Fig. 22.1)—see Chap. 35). This dimension of the therapeutic process is aimed at creating the conditions for the participants to identify and verbalize their personal conflicts [5].

The content of these conflicts is expressed by means of capacities (basic, secondary, actual), which are based on and affected by an individual's lifeline and their culture of origin. Clinical reflection on the degree of the individual's integration within the group is aided by contemplating their self-perception ability (including degree of separation), impulse control ability (channeled outward or inward), and ability to connect (to themselves and others) and by acknowledging their internal object (Fig. 22.2) [13].

Interpersonal Aspect

The nature of group therapy provides an excellent means of accessing the interpersonal aspect (Fig. 22.3), which is present in each dyad of group members. The cognitive and emotional insights into the content of interpersonal conflicts enable an individual to associate them with intrapersonal dynamics. The PPT framework offers a narrative that combines both dimensions in a manner therapeutically significant for an individual [11].

The interpersonal conflict in group therapy offers a chance to face and acknowledge the unconscious thought patterns and active capacities of interacting individuals (Fig. 22.4). Owing to open verbalization, it is possible to ascertain the content of internal conflicts that are reflected in the content of the actual conflict.

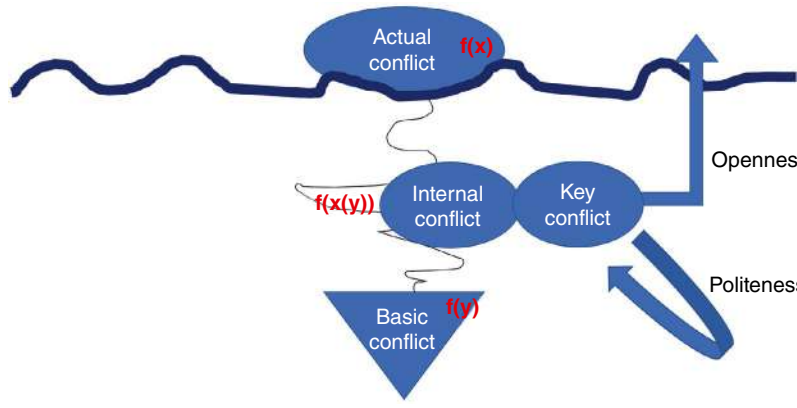


Fig. 22.1 Conflict operationalization

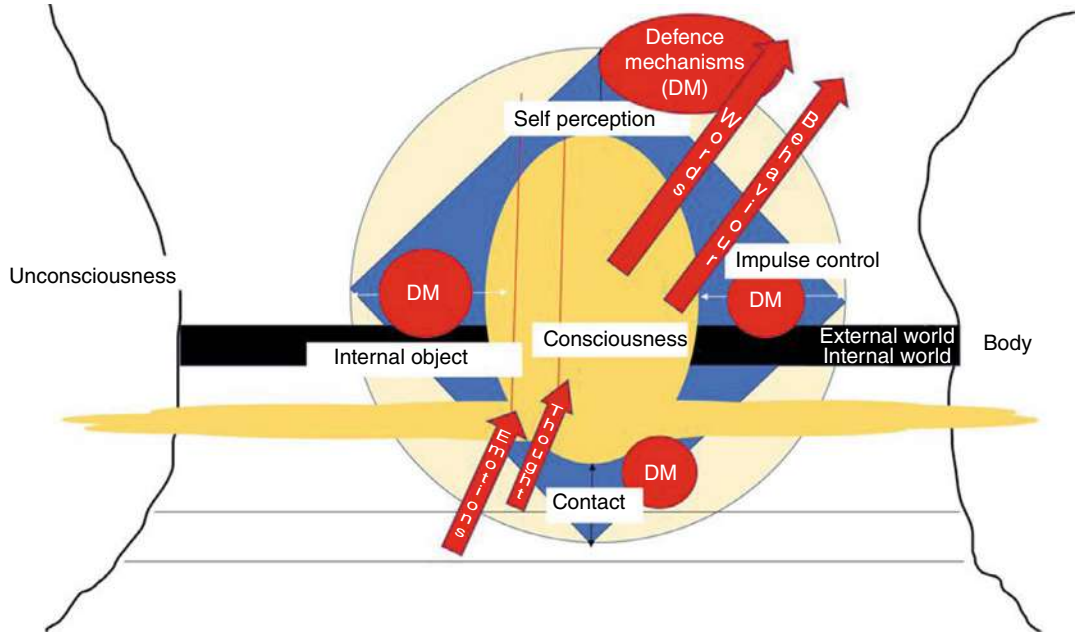


Fig. 22.2 Intrapersonal space

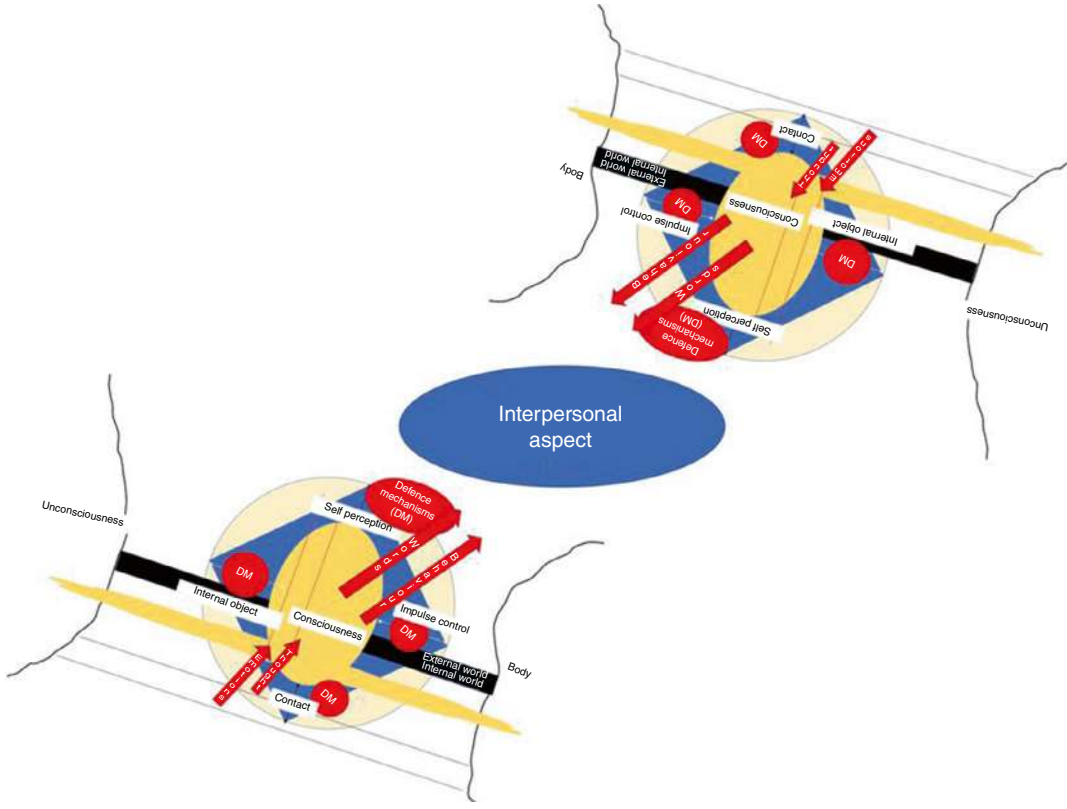


Fig. 22.3 *Interpersonal* space

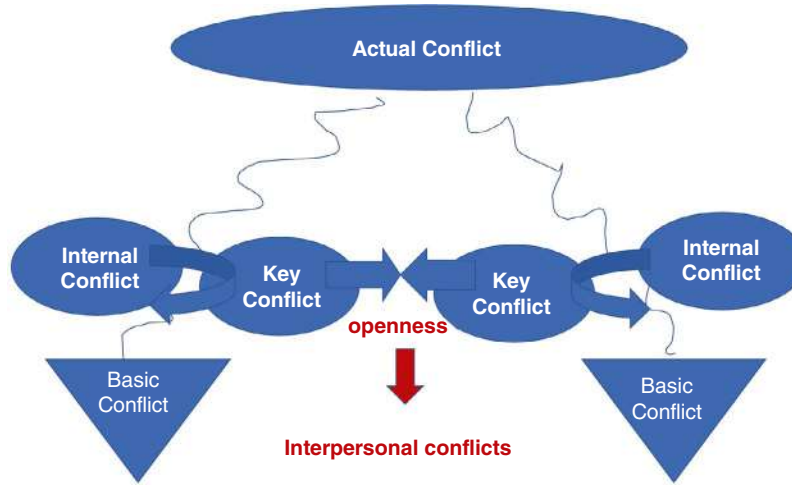


Fig. 22.4 Interpersonal conflict vs. inner conflict

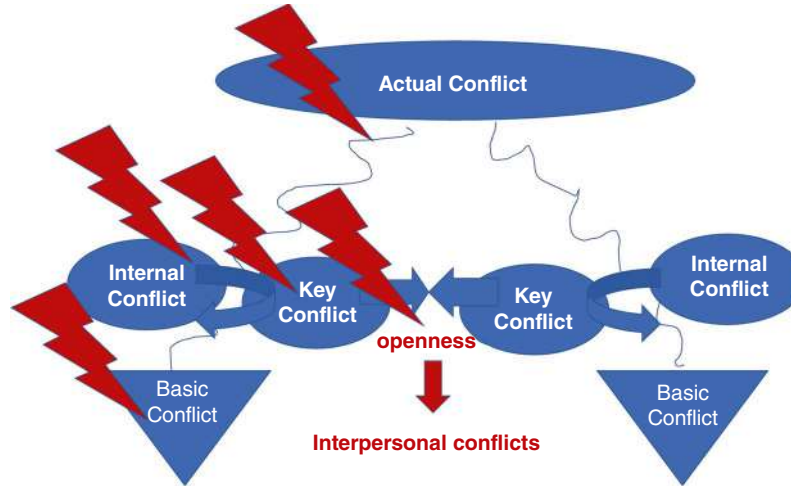


Fig. 22.5 The effect of cultural context on inner conflict dynamics

Positive psychotherapy emphasizes the aspect of cultural differences, just as the communication style, the content, and experience of actual, basic, internal, and key conflict is affected by the social and cultural contexts (Fig. 22.5).

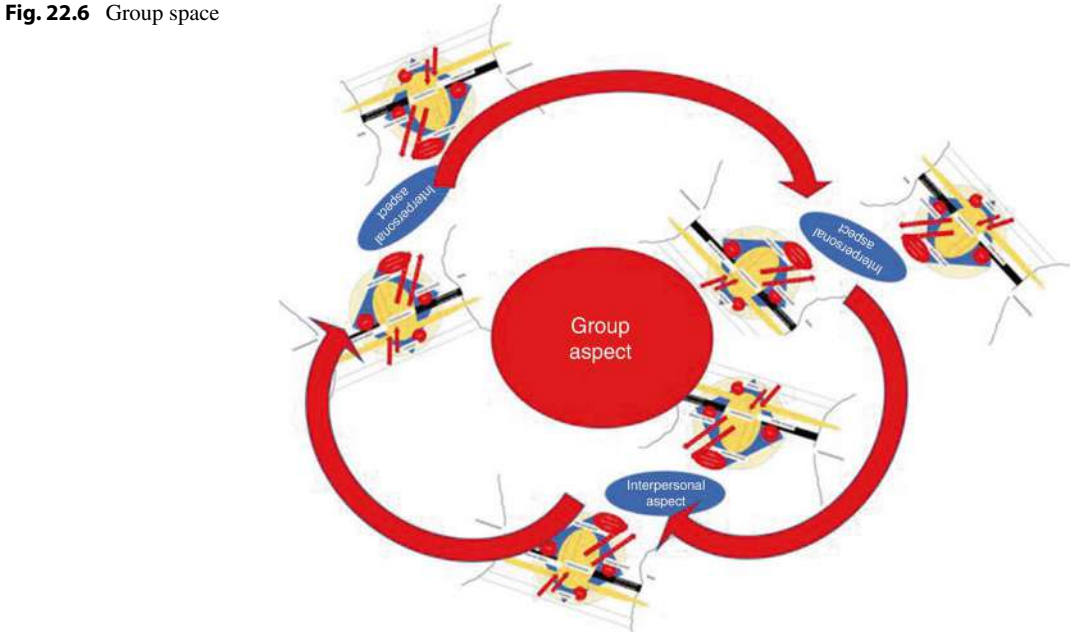
Group Aspect

Each group has its own conscious and unconscious beliefs and assumptions that apply to group task performance, decision-making, and roles, in addition to a set of formal and informal rules and norms (Fig. 22.6). The positive psychotherapist reflects on the phenomena occurring de novo in each group and on specific transcultural modifications brought in by participants belonging to minority communities (national, ethnic, sexual, autistic, etc.).

Systemic Aspect

Just like other social systems, a therapeutic system is complex and consists of multiple elements (people: participants and therapists; individual goals and expectations, reality, shared goals and the means of achieving them, place, time, space), being separated from the outside with an agreed, conventional boundary. Once the group is stable at the cohesion stage, the most balanced (multidimensional) therapeutic work takes place in the systemic area (Fig. 22.7).

Fig. 22.6 Group space



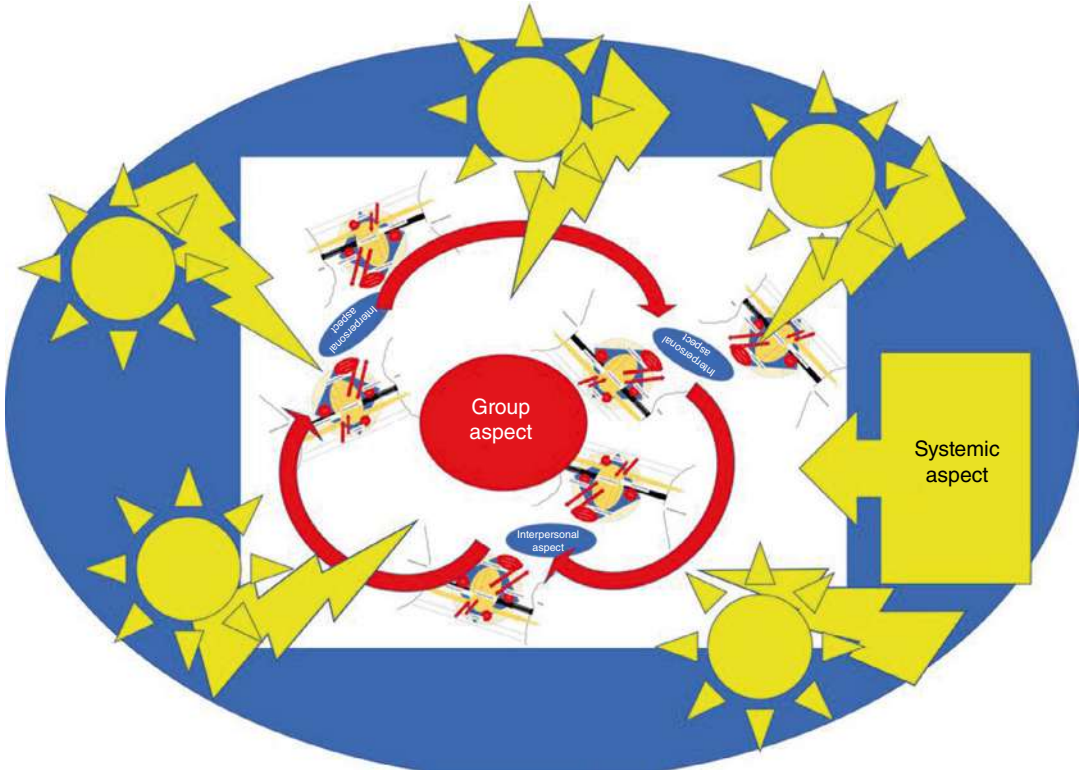


Fig. 22.7 Systemic dimension

Group Therapy Process

The term "process" denotes a character of interaction taking place inside the group, between its members and between the members and the therapist(s) [14]. It combines all the above-mentioned dimensions: intrapersonal, interpersonal, group, and systemic (Fig. 22.8). During group therapy, the therapist needs to reflect on both the content and the stage of the process. Metaphorically speaking, the process can, therefore, be likened to an operating system that serves as a background on which the participants' intra- and interpersonal space content is presented. Observation, clinical experience, and research carried out by theorists and practitioners representing different psychotherapy approaches have confirmed a certain sequence and repeatability of a group process. The course of individual stages and participant needs at these stages are universal and do not depend on the language of modality that is used for expressing therapeutic content.

At the *forming stage*, orientation, tentative conversations, search for structure, and dependency on the leader predominate the contents [14]. The *second stage* of group development, that is, *storming*, involves conflict and resistance. It is the stage at which the fight for control takes place and ambivalent, often hostile, feelings toward the therapist may be evoked. The actual therapeutic work is possible at the *third stage, norming and performing*. It is the stage of mature group cohesion, in-depth work on participants' goals, and their search for ways to meet their own needs in a mature manner. Each therapy needs a conscious closure, goal expansion, and shifting the therapeutic experience from the participant's resources to their reality. Therefore, I always emphasize the *final stage of transforming*, which completes the therapy process.

The role of the therapist, therefore, involves consciously supporting the transition through the subsequent stages and using the natural dynamics of this process as a means of working with the content brought in by the participants.

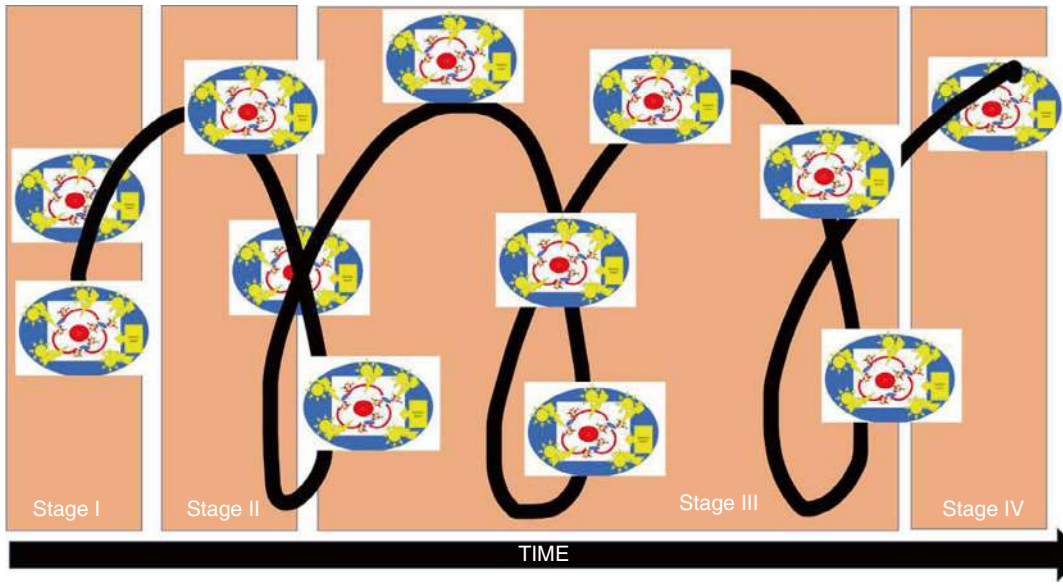


Fig. 22.8 Multidimensionality of a group therapy process

Application

Group Process Contents in Positive Psychotherapy

Having a multidimensional space, overlapping phenomena that occur in that space and time, a therapist supports group members in filling the space with contents consistent with their goals and needs. The PPT model has conceptualized the stages of a psychotherapy process (observation/distancing, inventory taking, situational encouragements, verbalization and goal expansion), and numerous therapy tools (described in detail in preceding chapters), which can have multiple uses in the hands of a therapist reflecting on the role of a relationship and a therapeutic alliance (Fig. 22.9) [3].

Considering multiple and various actions of a therapist during group therapy and different roles a therapist needs to play depending on the level of intervention, it seems reasonable to look at individual stages of group therapy in terms of their content in an attempt to conceptualize them using the nomenclature specific to PPT (Fig. 22.10).

Stage I

Group formation.

The earliest stage of group development is when the group structure is formed and working principles are defined.

- At the systemic level, the in-group social norms and the behavior and communication models are developed at this stage.
- At the interpersonal level, it is the stage of building group cohesiveness and trust.
- At the intrapersonal level, the participants feel the need to belong and determine their own role and place in the group.

The *key conflict* that becomes manifest in participants at this stage is the one between kindness and openness. The concerns at this stage of group formation can be verbalized to a variable extent. They address four different dimensions: (1) intrapersonal, (2) interpersonal, (3) group, and (4) systemic.

1. The first set of questions expresses participant concerns originating from their intrapersonal space:

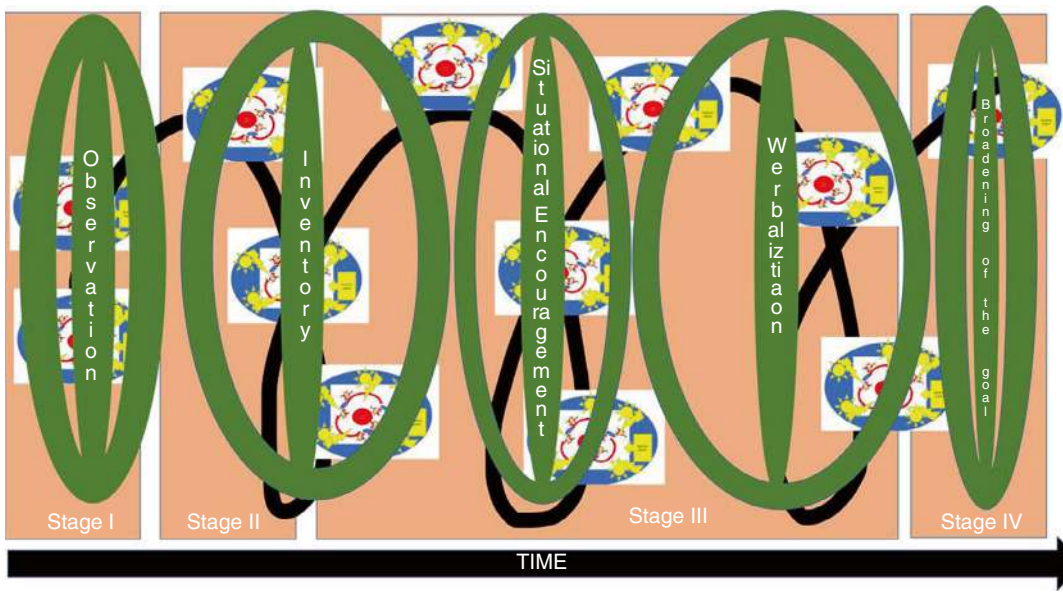


Fig. 22.9 Stages of a psychotherapy process and their respective contents

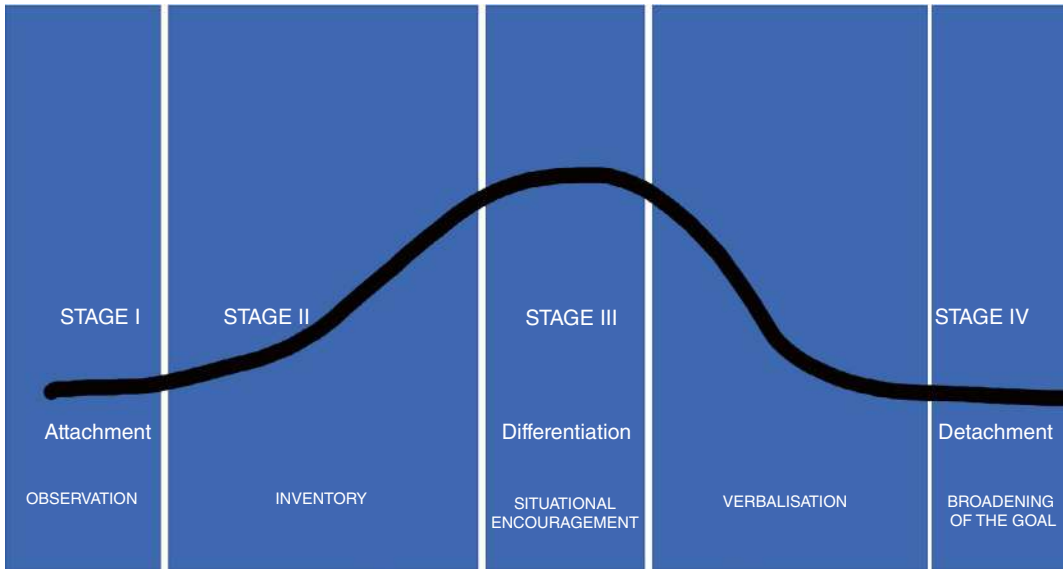


Fig. 22.10 Interaction stages in the group process vs. the psychotherapy process

- "Why am I here?"
- "How do I introduce myself?"
- "Can I safely disclose personal information here?"
- "Can I disclose who I am and still belong to the group?"
- "Will the group see me as a unique person?"
- "Will I not differ too much from others in the group?"

2. The second set of questions expresses participant concerns addressing the interpersonal dimension:

   - "Are there any other people like me in this group?"
   - "Will they understand me?"
   - "Will anyone want to listen to me?"
   - "How will they see me?"
   - "Will they accept me?"
   - "Will I be judged? rejected? ridiculed?"

3. The third set of questions pertains to the norms and principles of group therapy:

   - "What are we going to do during a group session?"
   - "What will it look like?"
   - "What are the rules and expectations?"

4. The fourth set of questions addresses systemic aspects of group therapy:

   - "Will anything I share be disclosed outside the group?"
   - "Will anybody else be able to access my records?"
   - "Will the participation in the group affect my future life, employment, etc.?"

At this stage, a therapist should facilitate relationship building and address the concerns arising in all the above areas. A therapist consciously models communication methods, supports the

definition of group norms and principles by creating the space to agree the contract, educates the participants on the basic phenomena of the group process, and provides a structure for group work. The amount of structure provided needs to be adjusted to create a balance between the need for structure and discouraging dependency. The main task for a therapist, however, is to foster the atmosphere of trust, in which participants can dare to be open and which fuels the processing dynamics of each participant at each of the abovementioned levels.

Stage II

Conflict and resistance.

The main challenges at this stage include the issues of power, authority, and control. The participants begin to reflect not only on what they have in common but also in what ways they are different. They feel the effect of a group process on their internal experience and have to confront their differing active capabilities. The first actual conflicts emerge. Group processes simultaneously take place at (1) intrapersonal, (2) interpersonal, and (3) systemic levels.

The challenges encountered at the intrapersonal level at this stage include:

- Recognition of one's own emotions.
- Readiness to express difficult emotions.
- Readiness to disclose and probe their own responses to what goes on in the group.
- Readiness to work on rather than avoid interpersonal conflicts.
- Promoting/reinforcing autonomy and independence.

The challenges encountered at the interpersonal level at this stage include:

- Spotting differences in the significance of individual basic and secondary capabilities for group members.
- Acceptance and openness to acknowledge and understand the systemic, cultural or biological origin of observed variability.
- Teaching the participants to respond to behaviors of other participants without labeling them.

The challenges encountered at the group level at this stage include:

- Modeling behaviors being a direct response to criticism.
- Awareness of group role delegation.
- Modeling respect for other group members' resistance and constructive response to different manifestations of resistance.

The challenges encountered at the systemic level at this stage include:

- Preventing subgroup formation and expression of difficult emotions outside the group.

The biggest challenge the therapist encounters at this stage is the need to intervene in a mindful and sensitive manner. Containing the group aggression directed at the therapist's role and undoing the potential setting up of a scapegoat can also be difficult.

The main task is to provide sufficient support for the differentiation process at its origin to become the first step of the inventory, which opens up a space for each participant to look into their own relational dimension and reflect on their individual anxiety-masking strategies. By fostering openness to expressing differences and dissatisfaction, the therapist consolidates participant awareness of group norms.

Stage III

The norming and performing stage can only be achieved after the two previous stages of group development have taken place and have been constructively closed. At this stage, the level of trust and cohesion among the group members is already high and the open communication enabling them to accurately express their feelings has been established. The feedback is spontaneous; it can now be received without being defended and accepted.

Using the nomenclature of the PPT, it is the stage for an in-depth inventory-taking, situational encouragements, and verbalization. At the intrapersonal level, the main goal for the participants is to identify their own basic conflicts, understand the dynamics of inner conflicts, in addition to conscious and unconscious coping strategies. At the interpersonal level, the main goal is to acknowledge and resolve the conflict between politeness and openness, and to consciously manage actual conflicts, while reflecting on the diverse development of capabilities in each individual. The group and systemic levels of group functioning are usually the gauge of a therapist's skills. If a therapist often uses whole-group interventions at the preceding stages, the participants will be able to maintain their systemic function. At this stage, a therapist is perceived as an experienced leader rather than the highest authority. The group members are important to one another, and the system becomes a source of resources and offers the space for individual growth and personality integration.

Stage IV

In the reflection of a positive psychotherapist, this is the stage for the closure of the therapy process by the expansion of goals and ending of the therapeutic relationship. The main task of the participants at this stage is to consolidate the learned contents and reflect on responsibility. A positive psychotherapist for the last time uses the tools specific to the modality they identify with to give the participants the space to reflect on:

- The degree of assuming responsibility for themselves (intrapersonal dimension).
- The type, extent, and limits to the responsibility for their spouse/partner, children, parents, other family members (interpersonal dimension).
- The extent of responsibility for friends, acquaintances, neighbors, other group members (group dimension).
- The responsibility of an individual for the world in which our lifeline has been placed and the human race (systemic dimension).

Entering the closure stage, the participants are confronted with the need to contain the emotions evoked by parting, which may, in some cases, require closure of unprocessed issues and problems. Separation anxiety may appear at this stage of a group process. The role of a therapist is to ensure that participants may express all emotions likely to occur because of the imminent completion of therapy and name potential areas that require further therapeutic attention.

Clinical Application: Examples

Alongside scholarly knowledge and clinical experience, PPT also originated from Nossrat Peseschkian's personal life history. Leaving his homeland (Iran) for his new home (Germany), he encountered great cultural differences between collectivistic and individualistic societies. Considering these life circumstances, the basic dual question of "What do all people have in common, and in what ways are they different?" seems deeply emotionally understandable. This question became the research hypothesis and the foundation for further development of a psychotherapy concept and the concept of "positum" [2], from which the name of the modality originates.

The transcultural perspective and emphasizing a holistic view (positum) of both resources and functional challenges opens up the space for therapeutic work with those clients, who, being extremely distinctive, do not fit the standard settings of psychodynamic, systemic, or cognitive behavioral therapy [15]. A good example can be multinational or ethnic minority groups, in addition to groups of immigrants or individuals with autism spectrum disorder.

Clinical Example: Positive Group Therapy for Individuals with Autism Spectrum Disorder: Specific Transcultural Aspects

Autism spectrum disorder (ASD), as a non-neurotypical developmental pattern, affects approximately 1.5% (1:67) of the general population. The Peseschkian question, paraphrased for the needs of group therapy in the ASD population, may therefore be "What do autistic individuals have in common with neurotypical ones and in what ways do they differ from them?" Differences in functioning, emotional codes, communication, and cognition form a basis of a unique culture described by autistic individuals. Knowledge, understanding, respect, and openness to neurodiversity are basic conditions that a therapist needs to meet if they are to become a leader of a therapeutic group of ASD individuals.

> The differences in communication style in autistic people are much subtler than in deaf people, and often very hard to detect for an outsider. Yet, such differences clearly exist, as autistic people often report that they have very few problems communicating with and understanding people 'of their own kind'. One could, thus, speak of a culture: communication problems arise when the cultural border is crossed. —(Martijn Dekker, Autscape, [1])

Stage I

Group formation is based on the atmosphere of trust and belongingness. Life experience, numerous relationship traumas, frequent iatrogenic traumas associated with "therapies" an individual may have been exposed to as a child and adolescent, make it difficult for a therapist to navigate a group transition to the subsequent stage. It takes special awareness of a therapist to ensure that connectedness occurs and a relationship be established at the interpersonal and group levels.

Below are needs expressed specifically by individuals with autism spectrum disorder:

> My main expectation from group therapy would be a closed group, preferably with people I have already met outside the group, whom I consider safe. Totally homogeneous—only individuals with ASD and/or ADHD, without significant comorbidities, such as personality disorder or bipolar affective disorder, with a formal diagnosis of ASD in all cases. The presence of individuals with personality disorder or bipolar affective disorder would make it impossible for me to be a part of the group. I think this is why I have never decided to join any open group sessions, e.g., at the day mental health units —Joanna, a 40-year-old female special educator, Asperger's syndrome

> In group therapy, I would expect to have my distinctness respected in a group space (not perceiving my feelings as a joke, being mean or challenging someone's authority) and tailored goal setting, where my needs differ from those of other group members. —Weronika, a 25-year-old female political sciences graduate, Asperger's syndrome

> I would like a therapist to consider ASD as neurodiversity, without stereotyping, applied behavioral analysis and attempts to suppress autistic traits in group members. I'd like a therapist to be able to identify the resources of individuals on the autistic spectrum and support them in developing self-awareness. At the same time, a therapist should support the group in developing the ability to see one another and to talk about their difficulties. Above all, a therapist needs to understand the difficulties that individuals on the (autistic) spectrum may experience when identifying their own needs.—Agnieszka, a 30-year-old female psychologist, Asperger's syndrome

At the forming stage, the ASD participants of group therapy can benefit from, and be put at ease by, learning a clear structure of a therapy process (observation/distancing, inventory, situational encouragement, verbalization, and broadening of goals). Distinctive emotional codes, sensory sensitivity or language processing can be reflected on as different aspects of being biologically transcultural. By introducing this narration, a therapist opens up a space for interest in diversity, a willingness to know it and understand its role in the lives of individual participants. As a result, relationships between group members can develop and a transition to the next stage is possible.

Useful transcultural aspects of the PPT process for individuals with ASD at the initial stage are:

- A clear, well-structured therapy process.
- Language of imagery.
- Transcultural reflection on sensory, motor, emotional, communicative, or cognitive uniqueness of individual participants observed during the first stage.

Stage II

The conflict and resistance stage is usually a major challenge for a therapist, and is even more difficult in a group of individuals with ASD. This greater difficulty stems from the relatively lower number of available and trained useful communication strategies, concomitant alexithymia, and relationship trauma fairly common in this population.

Below are needs expressed specifically by individuals with ASD:

> I'd like the therapist to be vigilant and prevent the group from scapegoating anyone, which I happened to have witnessed once and I suspect that the scapegoated person was on the autistic spectrum, just as I am. In group therapy, I'd like to learn how to be a part of the group without losing my specificity. I'd like to know how to work in a group whilst not giving myself up, and without resorting to behaviors that are completely unnatural for myself. I'd like to accept and to be accepted. First and foremost, though, I'd like to be understood. I'd like others not to assign negative interpretations to my behavior, which I have experienced in the past from the therapist in a group setting. I also expect that group therapy does not turn into autism re-assessment. I want to be strengthened rather than questioned.—Elżbieta, a 30-year-old female psychologist, Asperger's syndrome.

> Starting group therapy, I'd be afraid that the therapist may be unprepared, with no idea how people on the spectrum think or perceive, or (which is equally bad and dangerous) that the therapist may be an ABA practitioner. I would also be anxious about the unpredictability and "over-spontaneity" of the process.—Joanna, a 40-year-old woman.

> (I fear) that whatever I disclose to others may be used against me; that I will happen to be in a group, in which I will be jeered at…—Olga, a 22-year-old female university student, Asperger's syndrome.

Positive psychotherapy, with its transcultural emphasis, provides tools that enabled interpersonal conflict to be looked at from the value-based perspective (Fig. 22.4). Analyzing the situation to understand it is a strategy commonly utilized by individuals with ASD when in a group. For them, understanding is a basic prerequisite essential for feeling safe. This sense of safety, in turn, is a basic prerequisite essential for the first attempts at emotional recognition and emotional code identification (frequently masked by alexithymia) to take place.

Useful aspects of the positive transcultural psychotherapy process for individuals with autism spectrum disorder at the second stage:

- Tools available for use at the inventory stage (differential-analytic inventory, Wiesbaden Inventory for Psychotherapy and Family Therapy).
- Language of imagery.
- Reflection on interpersonal conflicts expressed using basic and secondary capacities.
- Awareness of interaction stages (connectedness, differentiation/discrimination, and detachment) and their resultant dynamics.
- Transcultural reflection on observed sensory, motor, emotional, communication, or cognitive uniqueness of individual participants.
- Using transcultural examples.

Stage III

The norming and performing stage of PPT involves inventory, situational encouragements, and verbalization. At the level of the inventory, each group member can face their own model dimensions, relational aspects, and lifeline, with particular emphasis on macro- and microtrauma, wisdoms, capabilities, and how they developed. Situational encouragements, often disregarded by other psychotherapy approaches, are of particular importance when working with ASD clients. The differences, for which they were ridiculed, suppressed, or humiliated for years may in their positum constitute the largest resource of an individual. Verbalization is the culmination point of each psychotherapy process. It is the moment in which each group member has acquired sufficient insight into their intrapersonal dynamics and has developed the resources necessary to integrate their personality structure further.

The above general description of the norming and performing stage of psychotherapy is true for any group. Working with individuals with ASD at this stage requires understanding the specificity of their perception and making sense of their experiences. Only after these particularities have

been catered for and embraced in the process will an individual be able to build on their therapeutic experience and develop a growth-promoting narrative.

Useful aspects of the PPT process for individuals with ASD at the third stage are:

- Inventory stage tools—model dimensions, relational aspects, lifeline, family tree, wisdoms, balance model.
- Microtrauma and macrotrauma theory.
- Basic capacities.
- "Positum" and its consequence for self-narrative by an individual with ASD
- Verbalizing intrapersonal dynamics expressed as conflicts (actual, inner, key, and basic), which combines the active capabilities one develops throughout ones' life, in addition to interpersonal dynamics.
- Transcultural reflection on observed sensory, motor, emotional, communication, or cognitive uniqueness of individual participants.
- Verbalization with inclusion and respect for transcultural aspects.
- Using transcultural examples.

Stage IV

The end of group therapy is associated with two main aspects: reflection on the responsibility model and the ability to end the relationship (detachment). Individuals with ASD tend to be "all-or-nothing" in their responsibility models. Some of them may present with learned helplessness and submissiveness (often a by-product of behavioral therapies they have been exposed to), whereas others, usually those with high IQ scores, tend to be overly responsible. Both attitudes may render them particularly vulnerable and prone to abuse and suffering. Realizing this takes time and understanding how group and system dynamics work. Furthermore, the positive experience of group therapy may often be the first life experience of a safe relationship with another person for individuals with ASD. Ending the therapy may, therefore, be experienced as mourning and grief, which is another aspect that a therapist needs to reflect on and consider when conceptualizing a psychotherapy process.

Useful aspects of the PPT process for individuals with ASD at the completion stage are:

- Responsibility.
- Awareness of the final stage of interaction (detachment).
- Verbalization with inclusion and respect for transcultural aspects.
- Goal expansion stage: working out goals for the next weeks, months, and years following therapy completion.

Conclusion

Owing to a holistic view of an individual, multidimensional conceptualization of psychotherapeutic processes and transcultural mindfulness, PPT can be used in clinical practice with very diverse groups. Despite the obvious advantages of group therapy, regardless of modality, two main conditions should always be prioritized. These are group selection, participant consent, readiness, and willingness. As much as they are objects of reflection for therapists, they also happen to be institutionally disregarded.

> (in hospital) I participated in therapeutic activities alongside individuals with autism spectrum disorder, mainly boys, who lacked independence, basic understanding of their own needs, sexuality or diagnosis; who were neither in education nor in employment, living on benefits. Compared to them, with my totally different problems, I seemed to be at the other end of the continuum. Needless to say, as a result of such activities, my functioning suffered significantly. The group started dragging me down. —Weronika, a 25-year-old female political science graduate, Asperger's syndrome.

Group mismatch, that is, disregarding differences in intellectual abilities, degree of independence, and concomitant mental health conditions, is the most common factor precluding an effective psychotherapy process from taking place.

Many individuals present with some degree of tension and uncertainty between the need to protect their own identity, intrapersonal separation, and the effect of participation in interpersonal, group, or systemic processes. The causes and nature of this phenomenon are complex and mul-

tidimensional. It is particularly manifest in representatives of all minority groups: ethnic, religious, or neurodevelopmental [8]. The in-depth reflection and understanding of these aspects pose a particular challenge for therapists and the worldwide community in the twenty-first century [7]. It is at least concerning that therapeutic abuse, just like the examples demonstrated in the ASD community, is still reported in many parts of the world. Many ASD individuals have been exposed since their early childhood, against their will and without their consent, to numerous "therapies" aimed at their behavioral adjustment to the needs and expectations of a neurotypical world. This stems from insufficient knowledge and understanding of their developmental specificity alongside unconscious projections of society and therapists.

> My dream would be that specialists have sufficient understanding of a human, their personality and preferences, regardless of the autism spectrum, to actually respect all individuals. Thus, in a nutshell, my dream would be that nobody (i.e., no specialist) offered me group therapy ever in my life. (…). I obviously agree with what others mentioned before (matching intellectual level, no pressure to speak/participate, no patronizing, IQ-matched group and therapist). However, even a remark of this sort evokes in me a very strong response to minor social trauma, to all those obsessive attempts to prove that "loneliness is abnormal" and ill. I am genuinely sad that despite my diagnosis and my fight for my identity, I still cannot be understood—even by those on the spectrum—and, like it or not, I still have to suffer due to the fact that (my) unwillingness to participate in group meetings is considered not ok.—Agnieszka, a 41-year-old female IT specialist, Asperger's syndrome

Group therapy is one of the sharpest tools in the psychotherapy armamentarium. Just like a surgical blade in the hands of a surgeon, it may introduce irreversible changes to the lives of its participants, the effect depending closely on the knowledge, experience, and personal preferences of the therapist. Thus, although these changes can "cure" and improve the quality and comfort of life, it may well take years of medical therapy and/or psychotherapy to remediate their consequences.

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Positive Psychotherapy in Organizational and Leadership Coaching

26

Yuriy Kravchenko

Introduction

In this chapter, we look at the concepts and practices of applying the method of positive psychotherapy (PPT after Peseschkian) in working with leaders, teams, and organizations.

> The question arises: who is sick and who needs to be treated, a person who appears to be a patient, his family, his partner, his subordinates and colleagues, the society and its structure, politicians who want to represent his public interest, or someone else, called his therapist? –Nossrat Peseschkian (1987)

The approach described here is based on the developments of the Ukrainian School of Positive Psychotherapy over the past 25 years and on the experience of working with more than 40 large and medium-sized international and local companies (business, public, private, non-governmental), which number from 50 to 10,000 employees. Among many others are *Vodafone, NovoNordisk, BNP Paribas, VTB, Metro Cash&Carry, Luxoft,* and *SoftServe*. The author is grateful to all his individual, team, and organizational clients for being the best teachers.

To make it easier for the reader to navigate and absorb the content, the material of this part of the book is structured into five subsections—alluding to the classic five-step model of Peseschkian [55]:

1. *Observation. O*rganizational *p*ositive *t*herapy and *i*ntegral *c*oaching (OPTIC).
2. *Taking inventory. S*ystem of *p*ositive *o*rganizational *t*herapy (SPOT).
3. *Encouragement. "H"-o*riented *t*riangle (HOT).
4. *Verbalization. TRACK* system in organization.
5. *Broadening of goals*. Model *"WORLD"*.

Part 1. Observation

*O*rganizational *p*ositive *t*herapy and *i*ntegral *c*oaching (OPTIC).

Observation of the Organization Request Through OPTIC

When we first start working with a potential client, we suggest working with the OPTIC model, which is also:

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_26

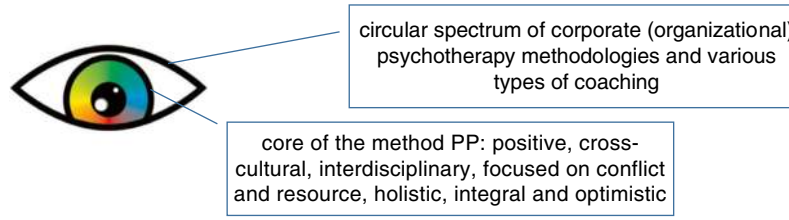


Fig. 26.1 The "OPTIC model" for observation in organization

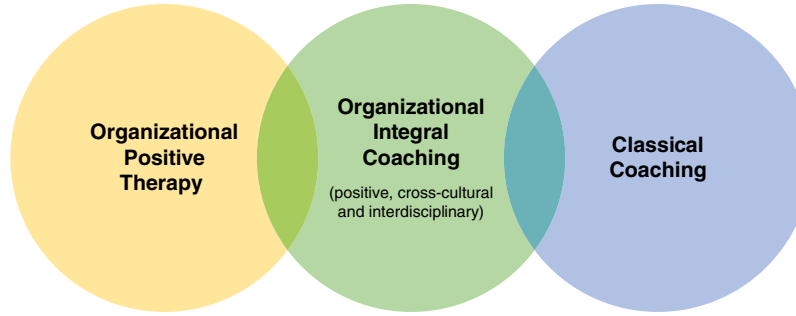


Fig. 26.2 Organizational PPT and integral coaching

A. An abbreviation of the name of the approach in which we will work: **o**rganizational **p**ositive **t**herapy and **i**ntegral **c**oaching (OPTIC) (see Fig. 26.2).
B. Strategic directions, which we will discuss during the observation: the organization itself, taking on the nature of its existence and its problems through the prism of the positive approach, possible prospects for the application of PPT models and/or integral coaching in this case.
C. A symbol of an eye, which is easily remembered and reminds us that we are at the observation stage with a client. In addition, (1) the "pupil" itself symbolizes a special vision through the core of the method: positive, cross-cultural, interdisciplinary, focused on conflict and resource, holistic, integral and optimistic; (2) the "iris" of the eye, which, as is well known, can be of different colors and shades, and is a moving muscular diaphragm, symbolizes a circular spectrum of various working ("movable," "muscular") methodologies, models and tools; (3) the "eyeball" area—that is, the eye itself as an organ—symbolizes the organization itself or its part that "sees the problem or task" and calls for help (Fig. 26.1).

Before proceeding to the working and applied sections, we briefly pay attention to the view—the positive view of human nature and organization (Fig. 26.2).

A View of Human Nature and the Nature of Organization in PPT

Positive psychotherapy focuses on human nature. Is a person born tabula rasa, or is there a certain predetermination in human nature? What other alternatives are there? And how does this affect our vision of large human communities, systems, and societies? These questions are raised by many ancient and classical philosophers in addition to thinkers and practitioners of the twentieth century: Freud, Jung, Adler, Fromm, Maslow, Rogers, and Peseschkian. Also, this issue is at the basis of the majority of

the world's traditional teachings and religions; it is an important reference point. At the same time, for example, in many of them, the detection, cognition, or discovery of one's true nature as a person is not only a reference point but the ultimate goal.

Positive psychotherapy offers a broad and at the same time both optimistic and realistic concept of human nature: we are born with two basic capacities: to love and to know. These capacities are an integral part of our nature, our essence, the innermost—which, through the development and discovery of potential, becomes explicit.

We come to the organization with the same capacities. Based on this, we view any individual person, or the entire human system (team, community, organization, society) through the prism of these two basic capacities.

Perhaps, at first glance, it seems simple or even simplified (the way many clients react at first). But then it turns out that this is a productive approach and it opens up new perspectives for a cross-cultural, existential, and integral understanding of organizations.

For example, for existential coaching in organizations, it is also important to understand the existential nature of people and organizations. In classical approaches, the human nature and its life are considered through "existential dichotomies" and "existential needs" (Fromm), "existential conditions/concerns" (Yalom), "existential motivations" (Frankl, Längle). This classic is good for existential psychotherapy, but for existential coaching it is too "cumbersome." A view through "love" and "know" (which means, through primary and secondary processes and capacities) makes it possible to describe dichotomies, needs, concerns, and motivations. For more information about existential coaching in organizations, see the section on existential coaching.

One of the tasks of a healthy organization is to help a person overcome the existential dichotomy of know/love and turn it into an existential dialectic. In other words, to create a dynamic integrity of know and love, in the "unity-in-diversity" of their differentiated manifestations through the self-realization of man.

Matrix of Work with Leaders, Teams, and Organizations

In this matrix, we use the scaling approach, which we have used since 2008 in the Organizational Coaching School and in the Corporate Training Program "Evolution of Leaders, Teams, and Organizational Systems" [38].

We work on three focused scales:

1. Individually with a leader and his life.
2. With a team.
3. With an organizational system as a whole (including leaders and teams).

At the same time, we can work at different levels of depth with each scale: therapeutic (deeper and longer) and/or coaching (more future-oriented and short-term in separate iterations; Table 26.1).

Table 26.1 Two-level matrix: coaching and psychotherapy in organization

	General review (can be realized by preliminary meetings or studies)	Individual work with a leader and his life	Work with separate teams	Work with an organization as a whole
Psycho-therapy	**At what levels does the *problem* exist?** *– Economic* *– Energy* *– Emotional* *– Existential* **.. How do we know that this problem exists?** **For whom is it a problem?** **.. And for whom is it – not a problem?** **– What are the cross-cultural views on this situation?** **– From what level and from whom does the REQUEST come?** **– At which E-levels are currently the largest resources?** **– What changes and results do we want to achieve and at what levels?**	**"Corporate therapy" as a psychotherapy in an organization** (conflict and resource-focused assistance from a specially trained specialist present in the organization)	**"Corporate therapy" as psychotherapeutic training on topics:** • Non-violent communication • Cross-cultural approach • Differential analysis • Five steps to solve problems in a team	**"Corporate psychotherapy" as a therapy of the organizational system at different levels:** • Perception and processing of current experience • Of the experience of present and past relationships • Conflict dynamics • Organization structures • Organization cultures
Coaching		– Managerial coaching – Life coaching and health coaching – Existential coaching – Vertical Development coaching – Perspective coaching	– Psychodynamic Coaching – coaching of existential motivations – Training coaching program "evolution of teams"	Systemic organizational coaching based on models: *4E model* *4R model* *Umansky model* *Bion's model* *Adizes model* *Cook-Greuter model* *SAPE model* *4M model* *five-step model* *DA-, TA-, EA- models* *(Kravchenk& Konischev, 2018).*

Part 2. Taking Inventory: System of Positive Organizational Therapy

This section is devoted to taking inventory—the deepest and most systematic part of the five-step process.

Here, we describe the characteristics and principles of the system of positive organizational therapy (SPOT) (Sections "Main characteristics of SPOT" and "Main principles of SPOT"), share the vision of working with the basic conflict in the organization (Section "Psychotherapy of the basic conflict of a person, team, and organization"), and summarize everything in one table, taking into account the modern developments of operationalized psychodynamic diagnostics (OPD-2).

We conclude this section with an example of the *first interview* that we use in working with organizations (Appendix: Organizational FIND: First Interview: From Needs to Development).

The classical method of PPT is described in the main characteristics and the three main principles. All of this is fully applicable to organizations. Further, we briefly clarify how the specific characteristics and principles of the method are refracted in working with organizations.

Main Characteristics of SPOT

Integrative Psychotherapy Method

- We consider everything that can or could be "mined" and developed using other methods

and approaches in organization: psychoanalysis, transactional analysis, management analysis, cognitive-behavioral approaches, schema therapy, gestalt therapy, spiral dynamics, "U" theory, training, etc. We do not denigrate other methods, but we try to integrate all of them, "gathering into one pattern." The integral map [68] also helps us in this.

Humanistic Psychodynamic Method

We simultaneously look "in two eyes" ("stereoscopic vision"—N. Peseschkian):

(a) We look positively at human nature, a person's basic and actual capacities and contribute to the actualization and self-disclosure of the potential of his/her abilities in the organization and in the workplace (this is the humanistic pole, the so-called "positive process").
(b) We see psychodynamics inside the personality, in teams, and in the entire organizational system, and we perceive psychodynamics not only in its form ("symptoms" and "syndromes"), but also in its content—"what is behind this?" (this is a psychodynamic pole, the so-called "substantial process"). Such stereoscopic vision allows us to remain both optimistic and realistic. Here, we use the best practice of humanistic-oriented psychoanalysts: Fromm, Kohut, and Winnicott.

An example of the practical implementation of these two poles of the method is the "psychodynamic coaching" developed by us (there is information about it later in the book). "Coaching" represents the humanistic, optimistic pole of the method, and the ability to see and work with psychodynamics—realistic.

Cohesive, Integrated Therapeutic System

- Positive psychotherapy is not only an integrative but also an integrating method that strives for the healthy integrity of any system. Based on two basic capacities, three principles, four areas, and five steps, we integrate, unite, and bring together the emotional, cognitive, behavioral, fantasy, physiological, relational, and spiritual aspects of the functioning of people, teams, and organizations. Integrative development results of other authors also help us in this [10].

Conflict-Centered Short-Term Method

- As already mentioned, we notice psychodynamic processes in an organization, not only in their forms but also in their content. Owing to the focus on the content and dynamics of the conflict (within the individual, within the team, or between teams or departments of organizations), the method works at the same time deeply, precisely, and in the short term. *"Do not repair what is not broken."*

Cultural-Sensitive Method

There are four scales here:

1. In a strict sense, we see that people working in the same organization are carriers of different ethnic, national, subcultural values, and we help leaders, managers, and human resource departments to take this into account.
2. Organization has not only a "body" (organizational structure and material base) but also a "soul" (organizational structure).
3. Organizational culture is a "system of systems," "culture of cultures," "transcultural," "transit culture"; cross-cultural processes take place within it, not only between cultures of different teams but, for example, between management culture and learning culture, between client culture and team culture.
4. Culture of organization, external culture of society, and culture of participants of the organization are in complex interaction and superposition [24].

Use of Stories, Anecdotes, and Wisdoms

- "Like golden apples set in silver is a word spoken at the right time" (Solomon). As modern organizations still remain very much

"left brain systems" (aimed at secondary capacities, logic, schemes, and figures), the use of images, metaphors, and creativity opens the door to new opportunities and resources. But it is even more valuable not just to "switch the cerebral hemispheres" but to connect them; using stories, anecdotes, and wisdom is best for this when the verbal (right) and the imaginative (left) hemispheres connect. For example, in team coaching, during business meetings, at strategic sessions, we can use parables, stories, myths, and legends as mirrors and as intermediaries [60].

Innovative Interventions and Techniques

- Colleagues developed such methodologies as psychovampirism prevention [50], stress surfing [29, 50], health coaching [55], decision-making methods, principles of personnel management, and management strategies [22, 25, 26]. And we further develop psychodynamic coaching and existential coaching to work with teams and leaders.

Application in Psychotherapy, Other Medical Disciplines, Counselling, Education, Prevention, Management, and Training

- The use of the method in organizations is already an application. But we strive not to make it one-dimensional: for example, having developed health coaching in the field of health care initially, now we are making a second arrangement: health coaching itself, connecting it, for example, with stress surfing, is transferred to the sphere of organizations and business [36].

Main Principles of SPOT

The three main principles or pillars derived from PPT and applied in positive organizational therapy are:

- *The principle of hope.*
- We have a humanistic hope that the human desire to realize the potential of its abilities and aptitudes is so great that even in one-sidedly developed organizations and human systems, people have many chances to show their positive nature. Organizational, labor, and business conflicts are sometimes particularly difficult and risky, but it is often they that give chances to learn partnership, openness, cooperation, and interdependence [53, 54, 69] and/or better understand the way they form and develop.
- *The principle of balance.*
- We consider the main balance to be the balance of development, renewal, and recovery of organizational structure and organizational culture, so that "hard" and "soft" processes in the organization are given management attention and time. We also work with an organization balance model and a differential-analytic model of capacities that it is important to harmonize at the situational and system levels, both in individual managerial or production relations and in the system as a whole. We have been developing this methodology since 2000 at the Ukrainian–German Center for Positive Psychotherapy and since 2005 at the Ukrainian Institute for Positive Cross-Cultural Psychotherapy and Management [22, 25, 26].
- *The principle of consultation.*
- Organizations, as complex organisms, being systems of interconnections [38], are wise

enough to regulate themselves in most cases. More often, people and teams in organizations do not need "repair" or "treatment" but accompaniment and support. One of the best forms, we believe, are events aimed at psychoprophylaxis and mental hygiene (measuring the mental health and emotional wellbeing of an organization), as well as coaching technologies (measuring horizontal and vertical development and new achievements). We build most of the technologies and processes on the basis of a five-step process, which is good for both consulting and coaching, and self-help.

Psychotherapy of the Basic Conflict of a Person, Team, and Organization

> You have to swim against the stream to get to the source.
> Go upstream to find the source.
> Nossrat Peseschkian

As basic conflict, in its broadest meaning, we understand the conflict between the past (which we carry in ourselves) and the present (which surrounds us). This basic conflict prevents us from moving forward to a better future that lies ahead of us.

We could organize this better future for us, if it were not for this basic conflict, "forcing" us to serve the past and turn the present in its direction.

This is a common pattern for individuals and for their families, communities, teams, organizations, and entire societies.

The basic conflict cannot be solved in a simple manner, for example, by changing behavioral habits, emotional acting out, negotiations, or balancing.

This is because the basic conflict includes the defense mechanisms and the coping strategies that proved their value earlier. These defenses and coping strategies are justified by negative events and traumas of the past (hereinafter, this corresponds to the first axis in) and protect against them. At the same time, they do not allow new potentials to be realized. There is a vicious circle: to protect ourselves from the worst, we avoid the best, and we repeat this psychodynamic cycle more than once.

In terms of in-depth approaches to coaching, the basic conflict refers to those situations where a person, leader, team or the entire system repeats protective psychodynamic patterns again and again instead of choosing to realize their economic, energetic, emotional, and existential potentials (realizing the potential in terms of the 4E system).

Positive psychotherapy is a psychodynamic conflict-centered method. Therefore, we pay much attention to the basic conflict.

At the same time, we do not consider the basic conflict separately but in combination with actual, internal, key conflicts and resources.

The method of positive psychotherapy at this stage of development offers a fairly developed technology of conflict operationalization [18, 20], which at the same time is consistent with the system of operationalized psychodynamic diagnostics [50].

This allows us to use this system in a flexible and focused way, both as tools for psychotherapeutic practice for working with individuals, teams, organizations, and communities as a whole, and through in-depth coaching (psychodynamic and existential).

Work System: OPD, Classical Method, Existential Expansion, In-depth Coaching Tools (Table 26.2)

Table 26.2 Work system, based on four axes of operationalized psychodynamic diagnostics (OPD)

Axis in OPD (first four axes)	Compliance in the classical PPT method	Expansion in positive existential psychotherapy method	Working practices with teams and organizations	In-depth (psychodynamic and existential) coaching tools
Axis *Illness, treatment, and health experience*	Positum-approach Cross-cultural approach to the interpretation of diseases and health, conflicts, and difficulties Positive view of the nature of a human and its capacities The principle of hope. "Global identity" (Peseschkian)	Introduction of the concept system: 1. Positive existential interpretation (*positive existential meaning*) [36, 43, 54] 2. Existential *capacities* [36] 3. Existential *concerns/ givennesses* [27, 69] 4. Existential *activities* [41] 5. Existential *identities* [23, 27] 6. Existential *motivations* [13, 43] 7. Existential *needs* (Ivanov, [36]) 8. Internal *existential objects* [36] 9. Existential *relationship* [36]	Work with criteria of "organizational" and "team" health. Positive existential interpretation of negative events (through an understanding of *existential motivations and meanings* [42, 61] What *existential capacities* allow you to develop these challenges and these givennesses/concerns	Psychodynamic health coaching of organizational system: work with the *disease-psychodynamics-coping-health* axis Work with four existential "R" in coaching: 1. What *result* will bring us the greatest *meaning*? 2. What are the existential *risks* of these *changes*? 3. What existential *resources* (activities, capacities, identities) do we have?) 4. What do we expect from new *resolutions*? What *existential needs* will they satisfy? What identities will expand? [36]
Axis *Relationships*	Differential relationship analysis Three stages of interaction in relationships Principles of hope, balance, and joint councils in relationship development	Relationship through identification with internal *existential objects* (heroes of stories bearing a positive existential charge), for example, Hodja Nassredin and Avicenna	Development of the *existential relationship* concept as an alternative to psychodynamic relationship *Existential analysis* of the relationship with the leader Defining *existential motivations* in teams	Establishing an *existential relationship* with a coach (including to counter psychodynamics in the client's systems) and their differential analysis using the systems of actual [55] and *existential* capacities
Axis *Conflict*	Operationalization of conflicts and their dynamics [18] Actual, inner, basic, and key conflicts	Consideration in the dialectics of *basic conflict* (emotional nature) and *basic resource* (existential nature) Resolution of the *key conflict* at the spiritual level [27]	What is the level: *Economic?* *Energy?* *Emotional?* *Existential?* of conflict? At what level is conflict resolution required?	Work with psychodynamic conflict in a three-dimensional coordinate system: 1. Organization structure 2. Organization culture 3. Relationships
Axis *Structure*	Four channels of reality learning and traditions fixed in them Basic emotional attitudes, entrenched in protective configurations	Existential identities and "five pinnacles of destiny" [23] Integration with spiral dynamics Integral approach	Structural maturity of the team and organization based on the Umansky model Practices of vertical development and ego evolution [3]	Check for compliance of the organizational structure with the organization objectives How do the leader's personality structure and organization structure interact? Coaching of perspectives [38, 39, 56, 57]

Part 3. Encouragement: Two "HOT" Models

Situational encouragement in PPT is a very important, key, core stage. In the word "encouragement," the base is "courage," and the words "core" and "courage" have a common origin. This is the emotional and energetic core of working with the client, the "hot spot" in the best sense.

To keep this in mind, we use two HOT models.

In the Section "Substantive "HOT" model: "H"-Oriented triangle", we describe a substantive model showing which topics and contents it is important to build interaction with the client around at this stage.

In the Section "Process "HOT" model: holders opinion trialogue", we touch upon the process of such an interaction: with whom and how is it carried out?

Finally, in the section, we refer to our practices on health coaching based on PPT.

Substantive "HOT" Model: "H"-Oriented Triangle

There are three central resource substances in working with leaders, teams, and organizations (Fig. 26.3):

1. *Hope.* This is a humanistic dimension and it corresponds to the first principle of PPT. *What do you have hope for? Which hopes are justified and which are not in this organization? What strengthens or weakens hope?* (more details can be found in the second section on the primary interview for organizations).
2. *Health*. This is an important focus, especially for corporate (organizational) psychotherapy. Considering health in a narrow, broad, and comprehensive sense, we are talking about team health and systemic organizational health. We are talking not only about physical health but also mental health, emotional health (emotional well-being), existential health (for more details see health coaching publications, where we consider various types of health: [32–34, 37, 38, 40, 45, 46]). Questions that are helpful to consider: *What are we already healthy at? How do we experience this health at the existential, emotional, energy, and economic levels?*
3. *Habits*. Useful and constructive habits that already exist—an important available resource (positum)—on which one can and must rely. *What are we already good at? What we do not need to change? How can existing habits help us to develop new ones?*

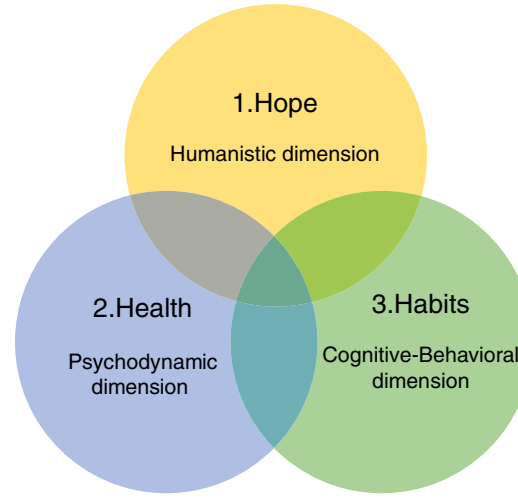


Fig. 26.3 Substantive "HOT" model: "H"-oriented triangle

We talk about these three focuses with an individual client or with teams, including strategic boards and boards of directors. In our opinion, it is important in the triangle of these topics to observe the balance of attention so that it is equilateral and we can take into account the *positive (humanistic), psychodynamic, and behavioral resources* of the client.

Process "HOT" Model: Holders Opinion Trialogue

The second "HOT" spot for encouraging and finding resources is to involve the maximum number of participants (holders) of the change

process in the dialogue. We consider at least three parties: (1) a customer of changes, (2) a client (he is not always a customer!), (3) a specialist who helps with changes (coach, consultant, or psychotherapist). If you bring them together, often their trialogue immediately switches to problems and solutions. At the same time, what is already good is overlooked.

> One rotten egg is remembered better than a thousand fresh ones
> Eastern wisdom

To involve these three parties in the trialogue exactly at the resource stage is a special chance to establish a solid basis for cooperation. Because when we proceed to solving problems, the dialogues and trialogues will be under a special test.

Part 4. Verbalization and Implementation of Changes

System of TRACK Principles

The fourth stage corresponds to the implementation and justification of changes. So that the interventions are systemic, accurate, directional, and safe, we have developed a system of five TRACK principles.

- *T—Training.* Change at the level of behavior is a necessary and sometimes sufficient level of change. PPT is a method of training for small behavioral changes in everyday life. This also applies to working life, business, and organizations. "One can open large doors with a small key" (N. Peseschkian).
- *R—Relations.* It is the psychotherapeutic perspective in organizations that gives an additional understanding of how important for the introduction of changes are changes in relationships and/or support in relationships.
- *A—Attitude.* Change at the attitude level is an important component, and here, PPT fully takes into account the groundwork of cognitive science and practice.
- *C—Consultation/consulting/coaching.* Everything described above (the therapeutic content of the changes) exists, best of all, and is carried out in an organization in the form of partnership counseling and coaching in the format of the relationship between two adult personalities.
- *K—Keys.* Corporate psychotherapists and consultants work in such a way that they become less needed, so that the "keys of self-help" (Peseschkian) are increasingly transferred to the client.

Each of these principles is taken from Peseschkian's method and reflects the essence of how PPT approaches changes. This is primarily a focused cognitive behavioral approach ("T" and "A" principles). At the same time, we are deeply convinced that changes at the level of behavior ("T") are mediated by relations ("R") —solely an attitude change is not enough ("A").

> Positive Psychotherapy is a relationship-oriented method
> (Hamid Peseschkian)

All of this is worked out and consolidated by consulting and coaching ("C") and remains with the client in the form of "keys of self-help" ("K"). This correlates with the third basic principle of PPT: the principle of counseling and self-help.

It is also important to note that in modern organizations a shift from management consulting to management coaching can be observed: there is a "softening" of the paradigm and a transition to agile management. Owing to the fact that since the 1960s, the consulting business especially has penetrated into an increasing number of countries, cultures, and economies; it has become not only a "knowledge industry" but also a "cross-cultural communication industry" [26, 30], all while progressive methods of psychology and psychotherapy have also moved in the direction of cross-culturalism over the past 70 years, in particular, the last 50 years. Peseschkian's method is one of these methods, and in the 2000s it was mature enough to become a deep cross-cultural methodological platform for management consulting, coaching, and corporate psychotherapy.

Further, in the Section "What is subject to change for a better positive future? structure, culture, conflicts, relationships, experience", we look at the levels at which we make changes with the help of corporate psychotherapy, and various types of coaching and training.

Then, in the Sections "The relevance and essence of positive psychodynamic coaching" and "Specific competences of a psychodynamically oriented coach and their focus on the elements of psychodynamics in PPDC", we describe one of the most interesting and modern types of coaching, which is becoming increasingly popular in organizations: a positive approach to psychodynamic coaching.

Finally, in the Section "Psychodynamic management in work groups: models and tools for team evolution and vertical development of leaders", we relate the management of team psychodynamics to their development and the development of leaders.

What Is Subject to Change for a Better Positive Future? Structure, Culture, Conflicts, Relationships, Experience

> Do not fix what is not broken.
> A small key can open a large door.
> (Nossrat Peseschkian)

> High-Quality Future is more accessible in High-Quality Present.
> (Inna Didkovska)

When we work with a leader, a team, or an organization as a whole, determination of the subject of our changes in coaching or psychotherapy becomes especially important.

What happens in relationships (for example, between a leader and a team; or within a team; or between teams)—is this normal or does it require intervention? How deep? Selective or systematic?

What should be done with the conflict? How deep and "healthy" is it? Or is it already destroying culture and structure?

Can we change the culture? Of a personality, team, organization?

How can the structure be changed and to what extent? Leader personality structure and organization structure.

This system of interrelated issues is complicated but solvable.

It is important to remember: what do we want to make these changes for? What better positive future? However, not only "for what?" but also "for whom?" What resources are needed for this? What risks await us along the way and as a result? Sergey Konischev, the trainer of the Organizational Coaching School [38], likes to ask clients and students at the School: "What's the worst which can happen—if you achieve the best results?" This question is surprising, and sometimes shocking. But in most cases it turns out to be useful.

Whether we work with an individual or with a human system, we treat these issues in a *differential and analytic way* [25, 53] on the one hand, and in an *integral way* [3, 67, 68] on the other. What does this mean in practice?

First, we certainly consider how deep we will "get" with our interventions on the following levels (Fig. 26.4):

1. *The level of current actual experience (actual conflict and actual resource):* its perception and passing through. For example, how does a team and a leader go through if, let us say, they are on deadline and it seems that the project will not be completed? What do they feel? What are they trying to do with this? How do they cope (the level of actual coping)?
2. *The level of relationship experience.* We often find here accumulated (past) experience with the actual tasks of reality and actual experience in the current relationships. For example, the team on deadlines in previous relationships with previous managers experienced fear—when managers began to "oppress" during times of crisis and threaten dismissal if the project is not realized on time. But now, for example, the new manager is trying to build a new experience in partner-coaching relationships. To do this, he listens a lot to the positions of the team members. However, this leads to inner conflicts among some team

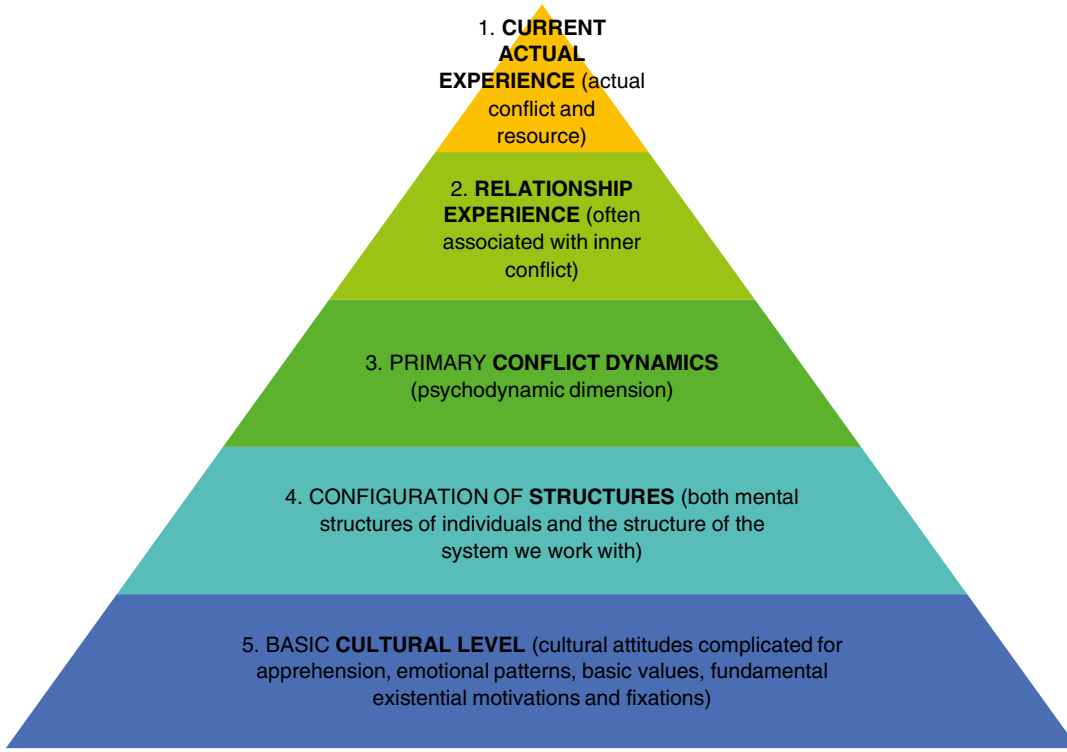


Fig. 26.4 The five levels in organization systems

members who cannot yet reconcile the accumulated experience of authoritarian relationships and the new partnership experience. These people seem to be torn inside between "obey and be afraid" (as it was before, and it somehow worked) or trust and begin to speak (taking risks, according to previous experience, to be punished for an opinion that does not match the boss's one). Then, we should help in accessible ways (coaching, facilitation, moderation) to realize and overcome this level of inner conflict.

3. *The level of conflict dynamics (psychodynamic level).* Here, laws come into force that can be constructively taken into account and understood, if one has psychodynamic thinking and a certain psychodynamic concept of the method. As PPT is a psychodynamic method, we have the opportunities to work at this level: through organizational (corporate) psychotherapy (if we want to work with the dynamics of inner and basic conflicts of the system and its participants) or through psychodynamic coaching (if we want to work with the dynamics of inner/actual conflicts, resources and behavioral changes).
4. *The level of structures.* Here, we are talking about the structures of individuals, teams, and organizations in general. This is a complicated level where interdisciplinary in-depth knowledge and careful practices are required [12, 25]. One of the main questions is how do the leader's personality structure and organization structure interact? What, why and how can we influence? It is also possible to work at the psychotherapeutic and deep-coaching level (psychodynamic coaching, existential coaching). Consulting is possible and managerial decisions are needed. But in any case, much depends on the productive orientation of the leader's personality structure [14, 16], his mental health and emotional well-being.
5. *The level of cultures.* In our opinion, this is the deepest level, because cultural content is pres-

ent in the individual and the collective psyche at the level of artifacts, heroes, traditions, and practices [53–55, 64]. First of all, it is important to respect, consider, realize, and integrate this level, and only then try moderately to influence it. "Old habits, like old friends and guests, should be politely escorted through the front door, and not thrown out the window" (Nossrat Peseschkian). Such work at the cultural level is possible owing to the cross-cultural nature of the PPT method and the possibility of working with value structures using existential coaching [50].

The Relevance and Essence of Positive Psychodynamic Coaching

Positive psychodynamic coaching (PPDC) has been developing for only the last 10–15 years, starting during the second half of the first decade of the twenty-first century [31].

In our opinion, this is because the world is becoming increasingly dynamic, and the leaders, who constitute the bulk of coaching clients, are experiencing increasing stress in a complicated and sophisticated world.

Volatility uncertainty complexity ambiguity (VUCA) world is a cause of anxiety, stress, and inner conflicts, which can be accompanied by increased psychodynamics within an individual, within interpersonal relationships in private and in business life, in teams, and in organizational systems.

As psychodynamic coaching is as close as possible to psychodynamically oriented methods of psychotherapy (including the method of PPT, as a psychodynamic, conflict-centered method), it seems to us important to find common traits and differences between these two approaches. We do this in Table 26.3.

As can be seen, PPDC is an approach that, like many methods of psychotherapy, focuses on working *with the psychodynamic content of the conflict.*

Also, PPDC, in addition to other methods of psychotherapy (including PPT), also focuses on working with resources (of an individual, team, or organizational system).

However, the peculiarity is that PPDC is focused on working in the present tense, with the existing *inner conflict* (the inner reality of a client, an individual or a system), as well as the *actual conflict* (mostly the external reality of a client), only sometimes describing [70] separate

Table 26.3 Comparison of psychodynamic-oriented psychotherapy and positive psychodynamic coaching

	Psychodynamic-oriented psychotherapy	Positive psychodynamic coaching
Work with psychodynamics	*Yes*	*Yes*
Conflict-centeredness	*Yes*	*Yes*
Resource-centeredness	*Yes, but not all methods (PPT—yes)*	*Yes*
Focus on conflicts	*Basic conflict/inner conflict*	*Inner conflict/actual conflict*
Attention to three tenses	*Past (50–80%)* *Present (50–80%)* *Future (0–20%)*	*Past (0–-20%)* *Present (50–80%)* *Future (50–80%)*
External/inner reality of a client	*Inner reality > > external reality*	*Inner reality ≤ external reality*
Briefness	*30–100 sessions (on average)*	*10–30 sessions*
Special competences of a specialist	*Psychodynamic thinking* *Psychotherapeutic relationship*	*Psychodynamic thinking* *Positive coaching world view* *5C competences*
Ultimate orientation	*Toward conflict resolution* *Resource activation (in some methods)* *Expansion of goals (in positive and existential psychotherapy)*	*Towards all 4R of coaching* 1. *Activation of resources (of an individual and system)* 2. *Risk management* 3. *Development of resolutions* 4. *Achievement of results and formation of further vision*

concepts and attitudes related to the basic conflict.

This allows PPDC to be oriented to the present and the future and to maintain a *balance of attention to the inner and external reality of a client.*

This also allows PPDC to be as short as possible (compared with short-term psychotherapeutic methods). Although it may be one of the longest coaching types.

Finally, the use of the classical structure of coaching (*4R model*) allows us not to get lost in the "wilds" of psychodynamic processes, phenomena, and psychological defenses. On the contrary: purposefully and vigorously enough to move toward a larger *resource*, good renewing *resolutions*, and a new positive *result*, taking into account *risks* through it all [38, 56].

But how exactly is psychodynamic coaching carried out? How is it not confused with the psychotherapeutic process? What are the differences between psychodynamic and classic coaching?

To answer these questions, it is better to look at PPDC from the perspective of the coach himself: what competencies should a psychodynamically oriented coach possess?

In PPDC, we use the 5C model of specific competences of a coach or a leader who meets psychodynamic processes or phenomena.

Specific Competences of a Psychodynamically Oriented Coach and Their Focus on the Elements of Psychodynamics in PPDC

Thus, we encourage coaches who want to be able to meet and work effectively with personal, team, or organizational psychodynamics, to develop their *5C competences*.

1. *Contacting.* In this competence, we mean not only "contact" as an event or a fact of life, but as a sensitive and not always sustainable process. And this contact may be interrupted and unstable. For example, psychodynamic phenomena and psychological defense mechanisms may interrupt contact. Therefore, we focus on the procedure of contact, as on contacting. In doing so, we prefer to rely on the *contact-in-relationship model* [10, 11].
2. *Containment*. Here, we use this term in a narrower sense than in the psychoanalytic tradition. This is the ability of a coach to withstand psychodynamic emotions, fantasies, threats, and intentions of a client in addition to his own. What is very important: this should not only be negative! For example, idealization at narcissistic psychodynamics is formally pleasing. However, it also requires containment, not a response [25].
3. *Confirmation.* Of course, we use this term beyond the religious meaning, and we understand by it the ability to confirm, validate, support, and assert the client's experience. At the same time, this does not mean that we agree with this experience. This is a confirmation of existence (positum—given, actual) and the value of experience, and the recognition of its subjective importance [10].
4. *Confrontation.* Oftentimes, this is the classic competence of both a number of psychotherapy methods and classic coaching in general. However, in PPDC we use confrontation very purposefully and in a focused manner (hereinafter we describe that it should be directed, first of all, at "criticizing and expecting forces," and second, at coping mechanisms, but with a view to limiting them, not destroying them; for this, the confrontation must be empathetic; [70, 71]). We follow a very important principle in PPT: the balance of confrontation and support. The confrontation is singled out in a separate, fourth, competence (only the fourth!). At the same time, support is the system of the first three competences: contacting, containment, and confirmation. Thus, we come to important balances in our method: the balance of "love" and "know," the balance of acceptance and justice, the balance of politeness and candor, the balance of the existing present and the desired future.
5. *Contracting*. This is the final, integrating stage, and competence at the same time. What

is special about PPDC is that it takes its time to bring the client to the contract (an adult and mature contract is hardly possible with active psychodynamics). The contract—in this case—is not the initial condition of PPDC, but its main product. Paraphrasing Freud, we may say the following about this important goal of PPDC: where there was an unconscious psychodynamic scenario, a conscious mature contract should appear.

We get not what we deserve,
But what we managed to agree about.

Psychodynamic Management in Work Groups: Models and Tools for Team Evolution and Vertical Development of Leaders

One of the important tasks in helping leaders, managers, and executives—from the side of a coach and a corporate psychotherapist—is to train them in psychodynamic management in work groups.

By psychodynamics, we understand the direction of mental and psychophysiological energy, cognitive attention not to achieve results and the search for resources and solutions, but to the scenario-based acting-out of basic conflicts, unproductive resolution of interpersonal conflicts, inadequate resistance to changes, and passive behavior.

The initial position that we offer to leaders and managers is that they themselves are partially responsible for the psychodynamics existing in the team. One of the key questions that we teach leaders to ask themselves is the following: *"How do I "push "my team to such a group psychodynamic response ("answer") with my habits, thinking, behavior, characterology, and perhaps my personal psychodynamics?"*

In addition, one of the key moments regarding how the psychodynamics of a team is connected with the personal psychodynamics of a leader consists in correlating the current maturity level of a given work group (team) and the leader maturity level. Here, we consider several possible options, which are frequent in our practice:

Option 1. *The leader is ahead of the team in its maturity*. Then we ask about what is happening with the team and how it feels that "the engine pulled away from the wagons"? Or, perhaps, they have a different situation? It also happens that the "engine" tears off "several wagons" and drives ahead with them, and the team ("the remaining wagons") continue to move by inertia and slow stops. Or do not move at all. In this case, the psychodynamics of the team (psychodynamic phenomena, processes, and manifestations) is very often a kind of appeal to the leader to turn around and go back for the lagging team or its part. It is like a child's cry by means of which he draws the parent's attention and calls for help. Very often, this may have the pattern of a self-defeating ("masochistic"), depressive ("sacrificial"), histrionic ("hysteroid") psychodynamics. In Bion's model, this may correspond to the basic assumption of dependency [25].

Option 2. *The team is ahead of the leader in its development.* In this case, we are particularly interested in what the leader understands, thinks, feels, and does. And also what he fantasizes about. If he is afraid that he will not be needed by the team. Indeed, it happens that the team, having reached the "autonomy" level of maturity, according to the "*Umansky model*" [38], expects the leader to be a "strategist," according to the Cook-Greuter model [39], because the team requires exactly this. But if the leader is at the lower evolutionary stages of his managerial logic development (for example, at the level of "diplomat" or "expert," which is good for "association" and "cooperation"), then the team can:

A. Devalue such a leader (narcissistic psychodynamics).
B. Blame such a leader (paranoid psychodynamics).
C. Morally "crush" such a leader or declare war on him (sociopathic psychodynamics).
D. Encapsulate so as not to interact with such a leader (schizoid psychodynamics).
E. Regress to the leader's level.

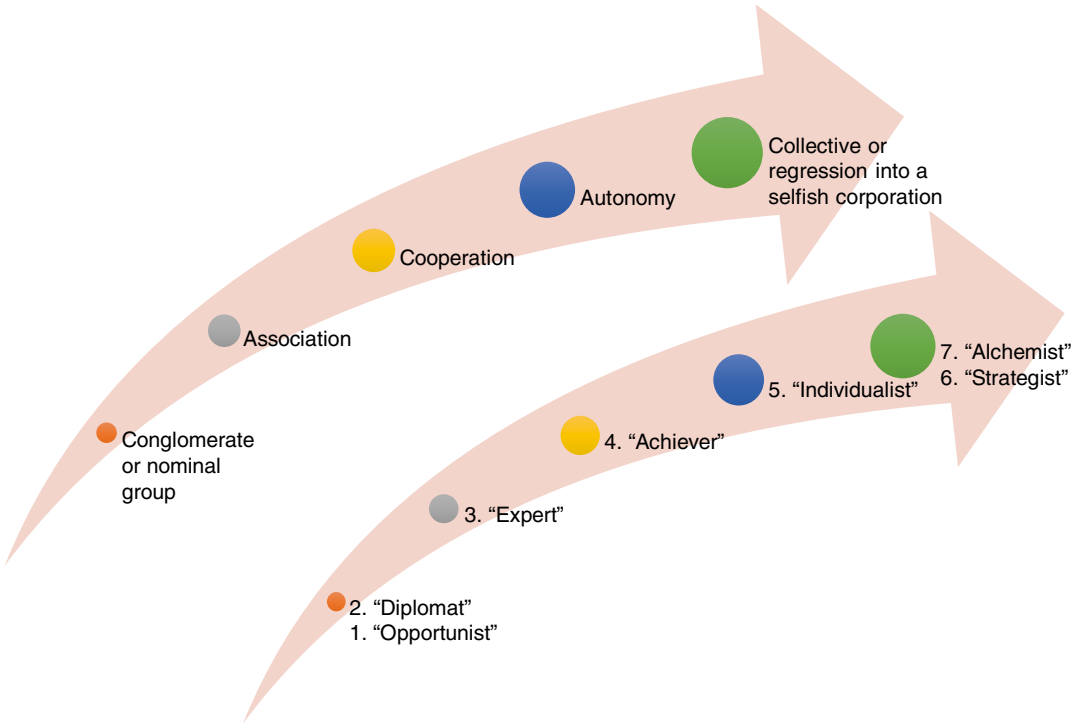


Fig. 26.5 Double evolution: team and leader

F. Go to one of the unconscious scenarios that correspond to the basic assumptions of fight–flight or pairing, according to Bion [25].

If such phenomena occur, then the coach will have to deal with quite complicated work with both the leader and the team. At the same time, it is good if the corporate psychotherapist can help both the leader and the team to at least be aware of their characterological peculiarities and defense mechanisms that support psychodynamics.

The main minimal task is to help to understand the current levels of evolutionary development of the leader and the team and help to relate them.

At the same time, the two main working models that we use for awareness and correlation are the *Umansky model* of “team evolution” (the upper vector) and the *Cook-Greuter model* of “leader’s ego evolution” (the lower vector). In coaching practice, we combine them visually in such a way as is shown in Fig. 26.5.

Part 5. Broadening of Goals in Working with Leaders and Organizations

Wisdom for Waves of Organizational Relationship, Leadership, and Development and Existential Coaching

In this fifth and final part of our chapter devoted to PPT in organizations, we will touch on the topics of new horizons, broadening of goals, and existential coaching.

At this step in working with the client, we again return to the mutual influence of “everything on everything” and what this means for the client’s future.

In organizations, we sometimes offer the “circles on the water” metaphor: how does what I do (or not do) affect my environment? Systems that surround me? Society? The world? The future? Traditionally, these questions concern leaders, but:

1. First, we believe that everyone is a leader at least for himself. Therefore, it is good to ask everyone these questions

   How can I affect the environment and serve humanity with my actual and internal situation and even with my basic conflict?
   Nossrat Peseschkian

2. Second, we note that there is something deeper than leadership; namely, the relationship. After all, leadership is a special kind of relationship

For practical use of these concepts, we use the Waves of Organizational Relationship, Leadership, and Development (WORLD) model (Fig. 26.6).

The "WORLD" model shows how, by changing attitudes and relationships in an organization, we can positively influence leadership. And thanks to leadership, "waves of water diverge"—waves of change, emotional and existential growth - into the dimensions of the development of people and teams.

Next, we look at how we can bring wisdom into the "WORLD" (Section "Wisdom of positive psychotherapy in leader evolution") at the level of the concepts and content of PPT, and then we describe the basics of existential coaching—our development—as the main tool at this stage (Section "Existential coaching: positive and positum approaches"). Finally, we conclude the section and chapter with closing thoughts that require further discussion and consideration (Section "Conclusion").



Fig. 26.6 Waves of Organizational Relational, Leadership and Development (WORLD) model

Wisdom of Positive Psychotherapy in Leader Evolution

When applying PPT in organizations, we attach particular importance to *vertical development*, the *evolution of leaders* [3, 11], *spiral dynamics* [1, 19, 21], and the *integral model* [67, 68].

The Cook-Greuter and Torbert models and the like are based on the understanding that people are developing, consistently rethinking the balance between differentiation and integration.

Therefore, this is in good agreement with PPT as an integrative method, and its differential analysis [55, 60]. At each level of evolution, the leader and the team have the task of integrating what was at the previous levels, and also learning to distinguish (differentiate) new content.

Wisdom is the ability to notice
Nossrat Peseschkian

The scope of this chapter does not allow us to describe all our experience and tools that we use. Therefore, we briefly described in Table 26.4 the correspondence between the level of the leader evolution and the PPT content: something that can be relied on to develop more and move on to the next stage, something that it is important to work with. At the same time, we metaphorically described each level as a special kind of wisdom (Table 26.4).

Existential Coaching: Positive and Positum Approaches

In relation to existential schools of the twentieth and twenty-first centuries [2, 4, 5, 13, 43, 44, 48, 61, 62, 65, 69], we apply an *integral, interdisciplinary, and cross-cultural principle*,

Table 26.4 Vertical development and identity expansion of leaders and systems

color codes of identity-levels:
Yellow: identities: **"I," "You," "We," "Prime-We"**
("Prime-We" is the greatest identity with the human race: our ancestors and descendants)
Green: identities: **"I," "You," "We"**
Blue: identities: **"I," "You"**
Gray: identy: **"I"**

Stages of ego development and logic of the leader's actions	A meaningful description of the development stage in the language of PPT and a special kind of wisdom
Uniting stage "Ironist"	"**Wisdom of ingenious simple souls**" (see Idries Shah) Actual capacity "integrity". The principle of "unity-in-diversity" We: **identification with humanity.** We, as a part of Prime-We Area: future/fantasy/spirituality. Me, You, We are parts of the world
Construct-conscious stage "Alchemist"	"**Wisdom of profound healers**" (Avicenna) Actual capacity "faith"/"meaning". Area: future/fantasy Identity: expansion from "We" to "Prime-We". Expanding global identity "The fifth pinnacle of fate" (V.Karikash) "I am a human"
Autonomous stage "Strategist"	"**Wisdom OF inventors for humanity**" (Da Vinci) Identity: "I," "You," "We": Identification with those who have similar principles, who are "like-minded". Positive thinking. Wisdom in conflict resolution. Development of global identity. A new understanding of the areas: work, contact and future
Individuality stage "Empathic individualist"	"**Wisdom of fair padishahs**" (see the Peseschkian's parables) Harmonious balance model as an exemplar Balance of primary and secondary capacities. Team is not as a mechanism, but as an organism. The development of cross-cultural sensitivity and respect for differences Identity- "I," "You" : the principle of "man for himself" (Erich Fromm) "I am "Ok," You are "Ok" Existential capacity to "be myself" and let others do the same
Conscientious stage "Rationalist" or "achiever", "successful leader"	"**Wisdom of honest scientists**" Differentiation of secondary capacities and development of primary ones Step: taking inventory/differential analysis Identity: "I," "You" (identification with like-minded people)
Self-conscious stage "Expert" or "technician"	"**Wisdom of expert technologists**" (example: IT engineers) Area: work/mind/logic. Secondary capacities "Me": self-esteem is based on the capacity to "know" Step: taking inventory/causal analysis Identity: "I," "Me," "Myself"
Conformist stage "Diplomat"	"**Wisdom of polite politicians**" Areas: contact/traditions and work/logic. Rational/pragmatic/adaptive use of contacts for profit. "Politeness" for the sake of safety and survival Step: observation Identity: :"I"/"Me"/"Myself"
Defensive stage "Opportunist" or "manipulator"	"**Wisdom of dexterous hunters**" Area: body/sensations/survival. Undifferentiated primary capacities. Deficiency of secondary ones Step: observation and distancing. The beginning of the tendency of separation from the merger (symbiosis of a newborn) Identity: "I," "Me" "Myself"

and of course, the *positum approach* toward existential concerns and a positive approach toward the existential nature of a person, first of all in relation to its existential capacities [35, 36, 47].

We add to these modern Western schools the existential wisdom of deep Eastern traditions: primarily Sufi wisdom and Zen Buddhist wisdom, in addition to the wisdom of Christian mystics [54].

In the key moments of an existential search for meaning, we also use concepts and wisdom, published by Nossrat Peseschkian in "In Search of Meaning" [59, 60]. In existential coaching, Peseschkian's views on the search for meaning are very organic for at least two reasons:

1. They are consistent with the model of Erik Erikson [6–9] and the further concepts of vertical development [3], the growing-up of adults, and spiral dynamics. This is very important from a coaching perspective!
2. They contain kindred (spiritually close) values to logotherapy and existential analysis of Viktor Frankl. This allows at the modern stage of development of the existential analysis of the "Third Viennese School" (Langle) their concepts to be integrated perfectly into our practice of positive existential coaching [36].

We fundamentally distinguish among *existential psychotherapy, existential counseling,* and *existential coaching.*

Existential coaching is the youngest of these three practices and aimed at healthy clients (individuals, teams, or organizations) ready and able to self-reflect and make existential decisions that change the course of life.

From our perspective, existential psychotherapy is more focused on the existential treatment of suffering and trauma, in addition to mature encounters with the ultimate concerns of existence. To be more precise, on existential maturation in the course of psychotherapy: *the development of existential capacities* [35, 36, 41, 42, 47] and *the expansion of existential identities* [23].

Existential counseling helps current life situations and events to be handled reflexively in one of four existential worlds [4, 5].

From the point of view of positive existential psychotherapy, existential counseling corresponds to the level of actual conflict, where simple cognitive-behavioral practices are insufficient because the actual conflict situation itself affects either deep areas (for example, the area of the meaning of life in Peseschkian's balance model) and/or is tied to one of the existential identities [23, 28].

A feature of existential coaching is that it remains in the *4R coaching space*: resources-resolutions-risks-result. It means that we stay focused on the result together with the client.

In existential coaching, we are talking precisely about the existential result. Such an existential result can include finding or creating meaning, making a vital decision, changing lifestyle, addressing the issue that will lead to greater inner freedom, discovering a new existential identity in oneself, transition to a new "pinnacle of destiny" [23, 28], resolving the key conflict [54, 55] at the integrative spiritual level [27]. Finally, the ultimate goal that existential coaching can focus on is to assist the client in the realization of his *fulfilled existence.*

Conclusion

The Concept of Conflict and the Concept of Development in Relation to the Person and Organization

Positive psychotherapy, on the one hand, is a psychodynamic method. This means that it is important to understand the dynamics of psychic conflict at different levels (basic conflict, inner conflict, actual conflict, key conflict).

On the other hand, PPT considers the development of a person throughout life as an important factor: first of all, through the differentiation of his/her actual capacities on the basis of basic ones, and also the inclusion of these capacities (I call this process integration) into four qualities of life.

At the same time, such a flexible and differentiated approach to development issues in PPT corresponds to the most advanced views in modern development sciences:

> … We also must keep in mind that maturation can be markedly uneven; that a person can have extraordinarily well-developed capabilities yet suffer from a crippling deficit in the area of, say, sexuality or the ability to be alone or the capacity to mourn, or comfort with competitiveness. "Fixation" is not a simple, unidimensional thing [49].

These two dimensions of PPT—conflict psychodynamics and continuing development through the differentiation and integration of abilities—are very important, scientifically, methodologically, and practically. We work within these two dimensions.

Integrity as a General Task of Overcoming Alienation

From the point of view of the actual capacity "integrity," the final primary actual capacity in the differentiation-analytic inventory [55] is the task of all types of coaching, which are described here. This task consists in strengthening and improving the internal reliable relations within an individual or a system of people (family, community, team, organization, society). Many researchers and practitioners claim that disunity and alienation is an ever-increasing problem [17, 59]. Coaching also seeks to overcome this problem [58].

Working with the Internal Enemy and Other "Internal Objects" in Psychodynamic Coaching

- When in positive psychodynamic coaching we meet with the effect and opposition of internal objects (Timothy Gallwey initially called them internal opponents), we do not fight them and do not oppose them. Even when they appear as forces (most often we single out criticizing, depreciating, expecting, and punishing [humiliating] forces) or parts, we still treat them as "living objects" and try to establish a regular, respectful dialogue. In this way, we somehow "humanize" these forces or parts when we begin to treat them "humanely": with attention, respect, fair evaluation, recognition of their value [42]. The latter can be successfully done through a *positive existential and cross-cultural interpretation* [36, 50, 52, 54, 56, 60].
- The ultimate goal of supporting nonviolent but structured dialogue is to invite the "internal heroes" to cooperate.
  - *Coach: Michael, what prevents you from starting to live your life more fully and making decisions that satisfy you?*
  - *Steve: It is my internal critic, internal opponent.*
  - *Coach: What is he saying?*
  - *Steve: He says that since I still couldn't put my life in order, now I'll also fail and I'll have to keep a low profile.*
  - *Coach: Steve, we may not investigate the origin of your internal opponent and won't fight him, we'll just get to know him better.*

Working with "Internal Objects" in Psychodynamic Coaching

In relations with the "internal opponent," we follow the recommendations of the schema therapy practice [71] and confront this internal object and its psychodynamics.

However, it is a short-term strategy that solves the here-and-now problem.

In the long term (systemic solution of the problem and the subsequent vertical development of the client), we suggest considering the confrontation of the "internal opponent" as the second of the four stages of emotional evolution [69]. According to the Weinhold model, we go through four stages in our development: codependence, counter-dependence, independence, interdependence. It is important to go through these four stages, in our opinion, when working with internal objects.

Two Levels of Work with Basic Conflict: the "Intimate" Core and the "Overt" Periphery

In the basic conflict, we consider two levels:

1. The phenomenology of basic conflict ("overt").
2. The essence of basic conflict ("intimate").

In PPT, it is important to respectfully accept psychological defenses and coping mechanisms as certainties. This means that we, believing in the positive nature of a person, trust what manifests and take it into account. We do not seek to immediately penetrate the "intimate." Working initially with the "overt," we gradually and

carefully move from the phenomenology of the "overt" (what we observe) to the content. There is a chance here to gain access to the "intimate" in the basic conflict. When working with leaders, teams, and organizations, this is especially valuable.

Versatility of Positive Psychotherapy as a Method in Organizations

We also give some important theses on the diversity of views on PPT

1. Positive psychotherapy as a "positive psychoanalysis." This method can work deeply with the unconscious level of leaders, teams, and organizations. At the same time, work is focused, safe, oriented on resources and solutions. That is why PPT is sometimes called "positive psychoanalysis."
2. Positive psychotherapy as learning wisdom through the ability to distinguish. The methodical core of PPT is differential analysis. In fact, we learn to distinguish between important details and nuances, and this is one of the facets of wisdom. We help leaders, managers, and teams to be not only smart but also wise.
3. Positive psychotherapy as a balance of differentiation and integration. PPT is an integrative method. At the same time, it is based on differential analysis. Thus, PPT has two balanced poles: differential and integrative. This coincides with the models of vertical development of leaders and allows us to use these models consistently.

	1. Overview	2. Analysis	3.Resources	4. Results	5.Broadening
My **Org**	O-1-1. **How do "capacity to know" and "capacity to love" appear in an organization?** O-1-2. **What can be said + and - about the organization rhombus?** O-1-3. **What is the organization's metaphor? What is it about?**	O-2-1. **What macro events in the organization's history represent it best of all?** O-2-2. **Which micro-events recurring during the year became micro-injuries and which ones contributed to well-being, success and health?**	O-3-1. **If, because of our organization, one more capacity appeared in the list of actual capacities, which one?** O-3-2. **If our organization is a "moveable feast," then what will it celebrate?**	O-4-1. **What are the main achievements and wise decisions of the company for the year?** O-4-2. **Who among us could become the most developing leader of the system?** O-4-3. **What should be changed?**	0-5-1. **How do I see the future of the organization in 5 years? How has my current vision changed for the last year?** 0-5-3. **What does our organization serve in the world? What does it develop?**
My **Team**	T-1-1. **Is my team more like an organism or a mechanism? What is it like? How does it work?** T-1-2. **If my team is a tribe, then according to what laws do we live? What traditions do we honor? Who or what are we fighting with? What would be on our coat of arms?** T-1-3. **What are our team hopes?**	T-2-1. **The team a year ago and now: DAO profile of capacities?** T-2-2. **What a.c. do we show well? Which of them have developed over the year? Due to what?** T-2-3. **What typical micro-injuries does our team get and which a.c. is it related to?** T-2-4. **What ways of conflict and stress processing are peculiar to us? Which are not developed?**	T-3-1. **What does our team like to celebrate the most?** T-3-2. **What resources do we have at each of the E-levels?** • *Economic* • *Energy* • *Emotional* • *Existential* T-3-3. What is positive about 3H? • ***H**OPE?* • ***H**EALTH?* • ***H**ABBITS?*	T-4-1. **What needs are there to change anything at each of the E-levels?** T-4-2. **What new ways of stress and conflict processing need to be developed?** T-4-3. **What are the first steps?** T-4-4. **Who/what will interfere/help? How do we cope?**	T-5-1. **Which "Side of the World" is our team moving to?** ***East:*** *community observancy. wisdom* ***West:*** *productivity. effectiveness, rationality* ***South:*** *contact. expressiveness emotionality* ***North:*** *thrift, restraint, vitality*
My **Self**	S-1-1. **Which of my hopes were justified, and which were not in this organization? What hopes are there left? What new ones were born?** S-1-2. **If my organization role didn't have a name, but someone who is smart and perspicacious watched closely what I was doing for a year, what would he/she call my role?**	S-2-1. **Where I move to concerning the organization - according to basic integral movements:** *1.* ***Merging with the organization:*** *I penetrate it and it penetrates me .* *2.* ***Separation from the organization****.* *3.* ***Differentiation****: I stay in contact and relations with the organization, and flexibly distinguish my identities.* ***4. Integration*** *: I move toward autonomous interdependence with the organization*	S-3-1. **What do I praise myself for? Top-5** S-3-2. **What achievements do I enjoy?** S-3-3. **What values am I fueled by and which needs of mine are being met in the organization?** S-3-4. **What capacities do I develop in the organization?** S-3-5. **When am I happy at work?**	S-4-1. **What do I personally add/ reduce, sow, cultivate, replant, so that my relations with myself and the organization become healthier, more productive, and developing?** ***in my Activities*** ***in my capacities(basic, actual, existential)*** ***in my identities***	S-5-1. **What kind of developmental experience does the organization give me, that I can now begin to become someone other (by profession, in life) than I was before?** S-5-2. **How important will work in an organization be when I am 75 years old?** S-5-3. **Which of these things will definitely be good when settling on other planets?**

Acknowledgments We are personally grateful to Volodymyr Karikash, a founding and leading figure of PPT in Ukraine, for the all-round support of our developments; Novo Nordisk, the Ministry of Health of Ukraine and the Ukraine Medical Academy of Postgraduate Education for the partnership; and the International Academy of Positive Psychotherapy (Peseschkian Foundation) for the recognition of our contribution.

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Psychotherapeutic Work with Men

27

Claudia Christ and Ferdinand Mitterlehner

Introduction

In recent years, more and more alarming newspaper articles have been published about the negative changes in the male self-image and the overall poorer psycho-physical state of male health [1–3]. The Cologne-based Zukunftsinstitut (Institute for Future) described men as "the psychosocial problem child of the 21st century" [4]. Other article headlines are "Reden oder saufen" (talking or drinking) or "women get sick, men die." On average, men's life expectancy in Germany is 7 years lower than women's. They have a significantly longer working life, sometimes work under very dangerous conditions (heat, cold, night shifts, manual labor), consume more alcohol and drugs, and are victims of physical, psychological, and institutional violence almost as often as women [5–8]. Mental illnesses such as stress-related depression [9, 10] or anxiety are rarely recognized in time and psychotherapy is still considered "uncool" for men. Why is everything surrounding the idea of "psycho" still so contentious? Or, to rephrase the question, what can positive psychotherapy (PPT after Peseschkian) do to improve the biological, psychological, and social state of male health?

For many years we have been working both theoretically and in practice to find out which psychotherapeutic offers men require, how these can be utilized in a constructive way, and how accepting help can be better integrated into the self-image of men [11–14]. How can and should a psychotherapeutic setting be tailored to the needs of men? We do not wish to divide psychotherapy into "female" and "male" forms of therapy, but even considering only a few factors can make access to psychotherapy much easier for men [15]. The starting point for men motivated to seek therapy seems rather intricate: If a man shows his distress and vulnerability, his "manliness" may be questioned by those around him. If a man does not immediately talk about his feelings, he could be considered unempathetic, cold, unable to trust, or unmotivated. More than 80% of all therapists are female, and even the setting of the therapist's office can be daunting for some "tough guys". Therefore, we wish to promote a gender-sensitive approach.

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_27

Biological, Psychological, and Social Health of Men

We can only think of health as the interplay between physical-biological factors, psychological resilience, and social inclusion [16]. In 1986, all the world's nations met in Ottawa to define the goal of "Health for All" in a globally accepted charter [17]. The following points are to be emphasized for men in particular:

- "Work and leisure should be a source of health for people. The way society organizes work should help create a healthy society."
- "Health promotion generates living and working conditions that are safe, stimulating, satisfying, and enjoyable."
- "Health is created and lived by people within the settings of their everyday life; where they learn, work, play, and love." (Ottawa Charter for Health Promotion 1986).

The charter shows that in every country, conditions should be created that make health possible in all areas of life. For men, this applies in particular to work, if it is performed under extreme physical or psychological conditions.

> This report by a construction worker is truly heartbreaking: For 35 years he had been slaving away at his job. Today, he is in disbelief, depressed, has given up on himself and is psychologically and physically exhausted because his pension payments will not be enough to cover his everyday needs. "All my life I have been toiling away, now I am left with nothing."

Many companies have recognized that they too have to care for their employees and that the implementation of occupational safety measures, healthy canteen food, low-threshold counselling services, back training, etc., is extremely important in lowering periods of incapacity to work in addition to reducing illness and premature departure of employees.

In addition to healthy situational conditions, however, one's own behavior is also decisive for a constructive long-term framework for safeguarding one's own health. Fast food, convenience food, coffee to go, or "to run" are all very bad sources of nourishment that cannot simply be offset by the occasional smoothie. Unfortunately, the food industry, as it is, is not conducive to getting rid of that excessive body weight. In fact, it promotes diseases of affluence, such as diabetes mellitus, high blood pressure, orthopedic disorders, etc. Drugs such as alcohol or nicotine only play into this pool of risk factors even more. With all that sugar, fat, alcohol, etc., our neuronal reward system is activated, and we cannot help but want more and more of it. In addition, media consumption turns us into couch potatoes, which is even more detrimental to our overall health [18].

The Balance Model in Positive Psychotherapy

The balance model according to PPT is an excellent tool for recording the interplay between the individual aspects of a particular biological, psychological, and social health status in a therapy session [19]. The model is easy to understand and visualize, does not put emphasis on educational level or native language, and quickly shows clear-cut areas of action for patient-related work. From this, therapeutic goals can be developed. The institutional, interpersonal, or intra-psychological aspects of these goals can be clarified and, if necessary, the therapeutic process can also be visualized. The balance model is also suitable for regular "self-assessments" even after therapy has been concluded [20]. "Are my personal pillars of life still in balance? Have I neglected individual aspects too much? Have recent events had a strong influence on individual areas of my life? In what way? What can I change?"

The first steps toward psychotherapy with the help of the balance model are low-threshold, structured, and comprehensive. Figure 27.1 shows an overview of the four areas of life "body/senses," "work/finance," "family/contact," and "vision/mission." This model can be quickly explained to both patients and clients and can also be used in the field of transformational coaching.

Once the balance model has been outlined, these four areas of life can now be expanded upon using specific questions. However, it is also possible to let the patient/client report freely and have them structure their own perspectives in accordance with the balance model. This gives the therapist a quick overview of the patient's life situation.

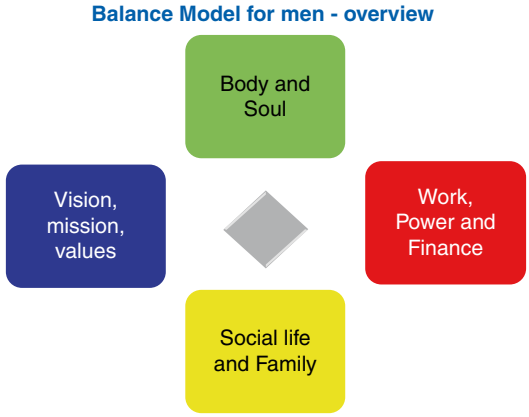


Fig. 27.1 Overview of the balance model according to positive psychotherapy

Expanding upon the four areas in a dialogue helps to create a biological, psychological, and social health model almost on its own and lets the client develop his own goals. For example, an "as-is state" for the question "What effect did your current situation have on the four areas of your life?" can be worked out. Next, a "to-be state" can be evaluated by asking the question "Where would you like to be in 6 months' time?" By reflecting on the therapeutic relationship, additional bonding patterns, maladaptive reaction patterns, and intra-psychological resistances can be reflected upon to bring depth-psychological and psychodynamic aspects to light.

The most important points of expansion that are tailored to psychotherapeutic medical work with men are summarized in Fig. 27.2.

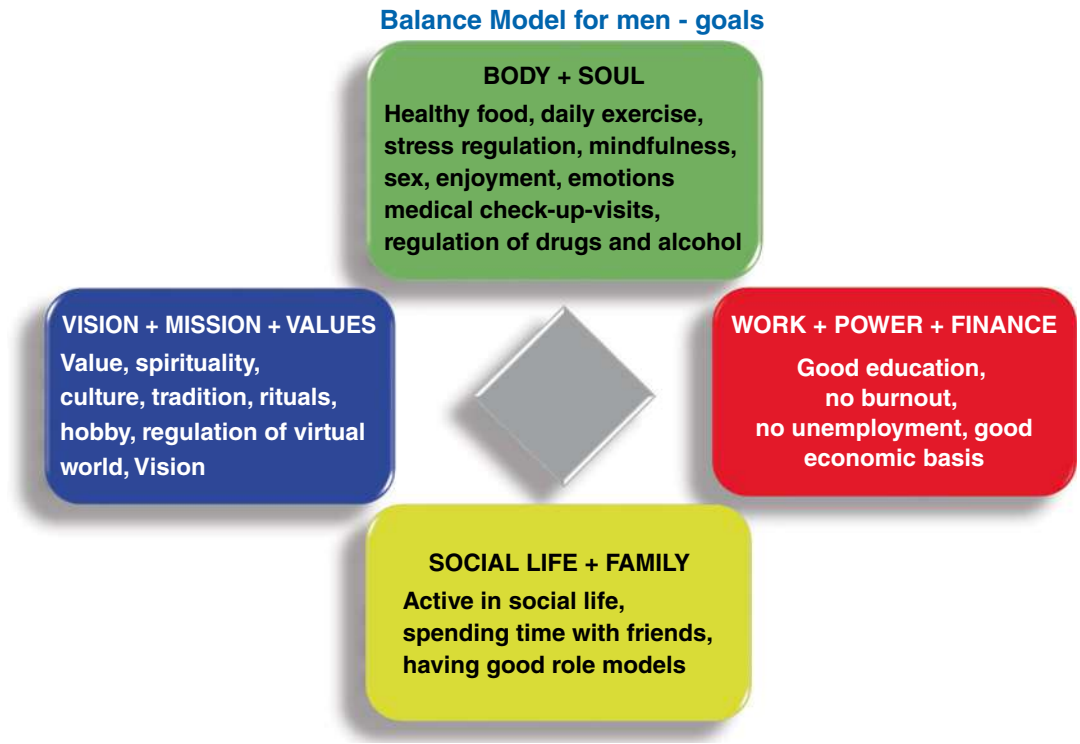


Fig. 27.2 Expanding upon the balance model for work with men

The Four Areas of the Balance Model: Key Considerations for Men

To enable specialized therapeutic work, this chapter presents the four areas specifically from a male point of view. Based on experience, men feel comfortable when the work to be done is outlined at the start of therapy [12], where even embarrassing issues can be discussed. At the same time, a classification of one's own topics as described by Aaron Antonovsky's concept of "Comprehensibility" [21] can be worked out. Therapy thus becomes apparently "easy" and low-threshold. Even after a few hours of therapy or coaching, the foundations for behavioral changes can be laid or the assessment standards can be redefined. For men, clarity on the choice of topic, the orientation toward everyday demands, the encounter with the therapist "at eye level," the desired help for questions about the working life, and the feeling that "there is someone sitting opposite me who may have experienced something similar" are particularly important.

"We suggest that good therapists for men, like good ship pilots, are informed and prepared" [22].

Balance Model: Body and Senses

Body + Senses
Healthy food, daily exercise, stress regulation, mindfulness, sex, enjoyment, emotional regulation, preventive medical check-ups, and regulation of drugs and alcohol.

"No Risk, More Fun": General Principles for Best Possible Physical Health

The commonly known basics for physical health such as healthy diet and exercise [23] have already been mentioned. Even if the therapist may not have been trained in orthodox medicine, it is important to consider the general medical and medication history to quickly recognize psychosomatic interrelations. Seeing a physician for cancer prevention, early detection of cardiovascular risk factors, sleep apnea, thyroid gland or blood sugar level check-ups, differential diagnoses of herniated discs, etc., happens more often for women than for men, as the business hours of the practices are often not compatible with the working hours of men. For this reason, the necessity of medical check-ups should be addressed during therapy.

> It is not uncommon for a generalized anxiety disorder to turn out to be the result of high blood pressure or hyperthyroidism, many times a depressive episode is caused by sleep apnea or low levels of certain hormones [24].

Fortunately, men often take on therapy as a result of a recommendation of a general physician or consulting physician. As Thure von Uexküll put it: "Nothing is purely somatic. There are only psychosomatic diseases" [25].

> Read what this 40-year-old university graduate who contacted the doctor's practice again and again after inflicting injuries onto himself said: *"It's happened yet again: I'm in too much stress and I've cut myself, sprained my ankle, injured myself during exercise, and so on. I wish to reflect upon my situation and change it."*

"Less Is More": Stress Regulation and Mindfulness

Many mental and physical illnesses are now classified as stress disorders. Many people see their professional and their private everyday life as permanently stressful. Companies in Germany are required by law to conduct a "psychological risk assessment" of their employees. In many cultures, the concept of "burnout" is socially recognized. At the same time, this phenomenon creates confidence among men seeking therapy. When labeled a victim of burnout, a man is allowed to say that the stresses he is put under are overwhelming and that the "personal limit" has been reached or they "just can't go on like this." We meet more and more young adults who are unable to cope with the pressures of our modern age and who seek psychotherapy stating symptoms of depression and fatigue. In particular, men who live in a single-earner situation tend to push

themselves too hard. Statements such as "everything is fine" or "everything will be fine eventually" are nothing but trivializing excuses for long-standing symptoms of stress. The very first warning signs of burnout or stress-associated diseases are:

- Sleeping disorders.
- Frequent infections.
- Back pain.
- Inability to focus.
- Social withdrawal.
- Erectile dysfunction.
- Aggressive or risky behavior.

Figures show that only half of all cases of depression (caused by exhaustion) are recognized and only a small percentage of them are treated adequately—a scary thought. Psychotherapy should therefore actively address the issues of stress regulation and body awareness [26]. In this way, individually meaningful measures for improving the regularization ability can be compiled. Even in a depth-psychological setting, men, in comparison, need more psychoeducational sensitizing and specific practical advice to translate the insight gained in therapy into everyday life. A positive attitude toward therapy and the intervention techniques of PPT are extremely helpful for this.

"Enjoy Your Sex": Sexuality

In his drive theory, Sigmund Freud outlined our drives or feelings of vitality and emphasized the importance of satisfying sexual needs and desires. As the discrepancy between the relevant theory [27] and the reality of sexual anamnesis created in psychotherapy sessions seems to be relatively large, therapy of men-related topics around sexuality should be addressed actively, without judgment and shaming. For instance, these sample questions could be asked:

- Are you happy with your sexual life?
- 20% of men (11% of women) look at pornography at work—do you?
- Would it be possible for you to live out your sexual fantasies?
- Do you suffer from erectile dysfunction?
- Are you happy with your gender role?
- What about homoerotic sexuality?
- Do you take any medications such as Viagra?

Case Examples

Filled with shame, a banker who developed a generalized anxiety disorder after a heart attack asked in a therapy session *"May I take Viagra again? I'm feeling inadequate toward my younger wife."*

Over the course of therapy, a facility manager at a hotel chain suffering from sleeping disorders, depression, and diabetes mellitus under insufficient glycemic control hintedat a subconscious trauma he experienced in his childhood. After he was given the chance to talk about it in therapy, the issue of regulating his blood sugar levels had resolved itself.

During therapy sessions with a young homosexual man it became apparent that he displayed self-destructive behavior by having frequent, unprotected sexual intercourse. Over the course of therapy, this behavior could be constrained successfully. "I was only looking for human closeness and comfort," he said after being subjected to confronting questions.

Since the idea of gender and sexuality is being increasingly accepted (or at least more frequently discussed), the "gender debate" is becoming more complex and thus the relevant issues for psychotherapy are becoming ever more relevant.

"Stop the Beginning": Addiction

> A 45-year-old journalist who is in treatment states: "I drink too much red wine and during stressful periods I smoke more than 40 cigarettes. I feel like I have damaged my own body to a degree that I am afraid I could get a heart attack at any time. I am also a notorious pessimist."

Addiction is a major problem, especially among men, and usually hints at more profound psychological conflicts, which can be accompanied by problems with self-esteem, childlike, regressive needs, or attempts at "self-medicating." Addiction is usually a creeping and therefore unnoticed process, which is associated with

the idea of "too little" (having received too little, being too little, experiencing too little approval).

- Addiction is a very slow process.
- Addiction means mostly longing for….
- "I need more" instead of "being more"
- Who am I? What shall I do? Where should I go?
- Losing yourself.
- Developing "tunnel vision".

There are countless substance-based and non-substance-based forms of addiction (Fig. 27.3). In addition to widespread addictions such as alcohol, gambling, and drugs, we are also seeing increases in addiction to physical fitness, healthy eating, and cosmetic surgery. The feeling of being inadequate, having to be perfect, needing to reach a certain professional goal can lead to tunnel vision, a weakening self-esteem, and can encourage addiction. In psychotherapeutic work with men, it is essential to address and reflect on possible addictive behaviors and implement health-promoting behaviors. The first warning signs of a possible addiction are as follows:

- Neglecting personal needs.
- Emotional anesthesia.
- Risky behavior (sports, reckless driving, sex).
- Conflicts with close friends or family.
- Avoiding feelings of anxiety and negative effects such as guilt, shame, and anger.

"Negativity and Hate Are Destroying Your Heart": Emotional Factors Contributing to Cardiovascular Disease Risks

In medical–physiological terms, we usually think of risk factors for physical illnesses as elevated sugar levels or changes in blood pressure. Research in the field of psycho-cardiology (i.e., questions about the possible influences of mental factors on cardiovascular diseases) has shown that emotional experience—just like risk factors that are measurable in the laboratory—has an influence on the calcification of blood vessels and

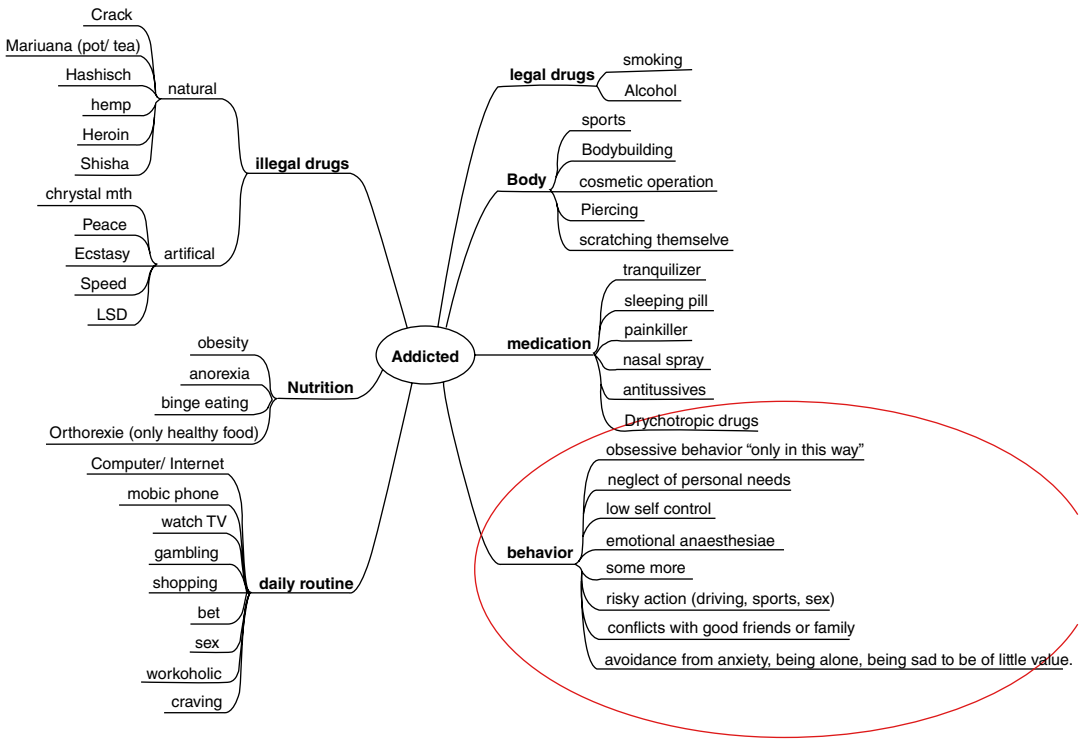


Fig. 27.3 Kinds of addiction: our attempt to gain an overview of the different forms of harmful dependence

therefore the pumping capacity of the heart. Experts also talk about "broken heart syndrome" when a heart attack is triggered by severe emotional stress.

Men also have "gratification crises" [28, 29], in which the ego, the self-esteem, the feeling of self-worth, the autonomous self-control, the dignity, and one's own effectiveness are very seriously violated.

Case Example

A 48-year-old man had to be taken to a hospital for 3 days after he was laid off by his employer. He suffered a severe heart attack and had to go into early retirement after the events. After being fired by his employer, his life had been turned upside down. Everything he worked for was over after he had received the letter of dismissal from his employer. After a long time and with therapeutic support, he was able to work out a comprehensive adaptation to his new life situation and was able to "recover."

The following emotional traits are "poison" for our cardiovascular system:

- Worse readiness.
- Perfectionism.
- Hate.
- Unfairness.
- Negativity.
- Rating.
- Competitive behavior.

During a recent therapy session, a 36-year-old patient sarcastically said: "In my case, you might as well tick all the items on your list!"

It is crucial to ask about early negative mentalization patterns in therapy and search for the origins of these thought patterns to develop new thought concepts with the patient. The patient mentioned above, who drank far too much red wine and had never reflected on his negative attitude before, was able to gain a more open view of his achievements in life and his own abilities.

Balance Model: Work and Finance

Work + Finance
Good school education
No burnout
No unemployment
Good economic basis

What boys enjoy is mostly forbidden.
Whatboysareespeciallygoodatisnotrequiredanywhere—not at kindergarten or at school.
(Quote by Wolfgang Bergmann, German family therapist and educator)

"What Boys Really Need": Good School Education

In addition to family, school is an important environment for acquiring the basics needed for social and professional integration. No phase in life is as formative and so full of development in the areas of motor skills, mental capacity, social skills, and hormonal developments as childhood and adolescence. These 18 years are a "rapid intoxication with all senses," which can both open new directions and close off many paths. At this stage of life, severe traumas usually have permanent consequences. Therefore, we would like to promote a healthy growth environment, both in family life and in school, which should cover the following aspects:

- Creating a safe, appreciative, calming, and stimulating environment that sets clear borders.
- A creative space for one's own experiences, mistakes, boredom, and frustration.
- Freedom for healthy psychomotoric development.
- Freedom to test physical strength.
- Learning through a general basic education.
- Learning through healthy and preventive behavior.
- Learning through craftsmanship.
- Learning through respectful social interaction.
- Learning values.

- Learning and adhering to rules and limits.
- Learning life skills and basic knowledge (i.e., legal, financial, ecological, etc.)
- Taking on responsibility early.
- Enticing development in the areas of music, art, literature, acting, etc.

Boys use school more as a meeting place than as an educational institution. Sitting at a school desk for 8 h is as detrimental to learning as playing 8 h of videogames. This turns boys in particular into so-called "educational failures." Forty percent of boys receive their general qualification for university entrance; in girls, the number is 60%. Ninety percent of children with attention deficit hyperactivity disorder are male. In modern society, it seems like there are virtually no children who are not assigned a label such as being "gifted," "highly emotional," "dyslexic," "dyscalculic," "hyperactive," "socially incompetent," or "allergic." Why do we no longer use family and school space as an environment for educating young people on how to be responsible, socially integrated, and well-educated?

"Be Happy With Your Work": Working

"Those who enjoy their work will have many happy hours in life" (a quote from a friend who has been stationed in Afghanistan for 9 years with the German military). There is nothing to be added to this quote. As we spend about 40 years of our life at work to make a living and save up for retirement it is important to make the most of our time there. The better the education/vocational training, the better the chances on the job market. Occupational identity gives purpose and contributes to personal integrity [30]. It is therefore logical to see that the transition from active working life to retirement age presents an individual challenge for many men [31].

The "wheel of work" [32], as illustrated in Fig. 27.4, gives a comprehensive overview of the different aspects of work and can be used to create a structured reflection. From a methodical standpoint, the transition between coaching and psychotherapy is certainly fluent. As work is a very important dimension in itself, especially in the lives of (single-earner) men, this model can be used over and over again for purposes of self-reflection. Asking explicitly about difficulties with colleagues or superiors, about financial security/insecurity, missing or received support from others, and about one's own motivation in the job, provides a concrete entry point for working on the topic.

> "My job has become completely meaningless to me" a 54-year-old marketing executive said a few months ago.
>
> A young trainee described his situation as follows: "My superior only criticizes me and I have no idea what I could do to improve."

This model can be used in therapeutic work to uncover intrapsychic issues, subjective attributions of meaning, social strengths/conflicts, and institutional influences in the working environment. Statements such as "I always get into a fight with my boss" possibly hints at underlying bonding issues or maladaptive relationship patterns. It is also possible to use this model in a transformation process as a method to get from an as-is status to a to-be status.

"Be Aware of Your Limits": Burnout

Our modern working environment demands a lot from us, especially mentally. Be it high flexibility, high pressure to perform, high-speed requirements, more stressful situations, fewer



Fig. 27.4 Wheel of work. (Model varies according to full circle global)

opportunities for retreat, constant availability, understaffing, more personal responsibility, etc. These stress factors can lead to psycho-physical exhaustion that, in turn, leads to clinical depression or a generalized anxiety disorder.

Issues at the workplace often cause men to seek psychotherapy for the first time [33, 34]. Another reason is that they are often referred by a psychiatrist or a general physician. As a therapist, it is important to encourage men to work on topics immediately and not to wait too long. Depressions, especially in men, can be trivialized as "all is fine" or be disregarded as nothing but "acting out behavior." In principle, the following aspects of therapy for men are to be taken into consideration:

- Men are downplaying their symptoms "All is fine!"
- Depression in men is often undiscovered—the symptoms are reckless driving, drinking, violence, aggressive behavior.
- Unemployment and a lack of recognition are risk factors for burnout.
- Increase in absent working days as a result of burnout/depression.
- Presentism (people not calling in sick when they should, the consequence being that they are only physically present).
- Absenteeism (people pretending to be ill to avoid working).

In our online contact form, people pick burnout as the reason why they seek treatment over any other illnesses.

Balance Model: Family and Contact

Family + Contact
Active family time
Active contact and social life
Spending time with friends
Being a good male role model

Family and Friends: Sense of Belonging

One of the possible basic conflicts of the psychodynamic diagnostic instrument OPD-II [35] is balancing between belonging and autonomy. Knowing that we "belong somewhere," have a certain place in society or, more broadly, are involved in society, gives us a sense of purpose and the feeling of being needed. The two questions "what do we want to be like" and "what are we supposed to be like" have to be answered by the individual themselves.

Global online social networks such as Facebook, Twitter, Instagram, etc., are more popular than ever before. Every way of life, every gender identity receives "likes" and is accepted. In a world where everything is possible, how can we know which way of life is the best for us?

The concept of "family" has recently become a heated battlefield, where legal, psychological, transcultural, and open aspects of family life are being discussed. One of the questions we ask our patients most frequently is "Who exactly do you mean when you talk about "WE"?" Who exactly is part of "WE"? Relationships are no longer self-explanatory (family = father, mother, child), but require a clear definition.

Belonging and social relationships must therefore always be re-established and resemble a perpetuum mobile rather than something fixed. Who accompanies us in life, who can we trust, who helps us in difficult situations? Oftentimes, the therapist is an important person of trust because this exclusive time of listening and him putting his own interests aside has become something special for the patient.

In the therapeutic setting, we ask not only about the biographical history of the original family but also about the "current family." In depth-psychology in particular, we want to get an idea of the early bonding patterns, the important persons of reference during childhood, the atmosphere in the parental home, and the satisfaction of basic needs to better understand the dynamics of the lives of our patients. Some fundamental questions of depth-psychology are how relation-

ships can be shaped and how tensions around crucial issues over the entire course of life can be resolved.

Who Am I?: DISC Model

The so-called DISC model (Fig. 27.5) is a simplified but quickly comprehensible model for patients/clients with different personalities and social relationships [36]. This determines, for one, whether we are people-focused or task-focused and also whether we are active or reflective as a person. In the DISC model, there are four main behavioral styles: *d*ominant, *i*nfluential, *s*teady, and *c*ompliant. As a rule, every behavior has advantages and disadvantages and we assume different roles depending on the situation.

> "I wasn't even aware I was so demanding in my interaction with people," explained a marketing manager, who, during our therapy sessions, gave me the constant feeling of needing to perform fast.

In this case, the borders of coaching and psychotherapy become fluid once again. We consider models to be helpful in explaining the complexity of the psyche to the patients so that they can become "experts of their own person."

Recurring, mono-dimensional relationship patterns, which other people perceive as disturbing and stressful, can then be worked on in axis 2 of the psychodynamic diagnostic tool (OPD-II) [35] in a therapeutic setting.

Being a Good Father: Men as Fathers

"The most powerful role models for children sit across from them at the dinner table."

Men are different from women and fathers treat children differently than mothers do. And it is fine that way. Studies show that fathers who work more than 55 h a week and are not available to the children are not regarded as a person of reference by them. This creates above-average numbers of behavioral abnormalities in the sons [37]. Spending quality time with one's children in addition to the partner is an important factor in making role models effective. In a therapeutic setting, we ask men about their experiences and memories with their own father. Sometimes, working with a family tree is also helpful to discover certain patterns in family history. Which male role model of the patient becomes apparent? What kind of father do I want to be? What kind of role do I want to assume as a partner?

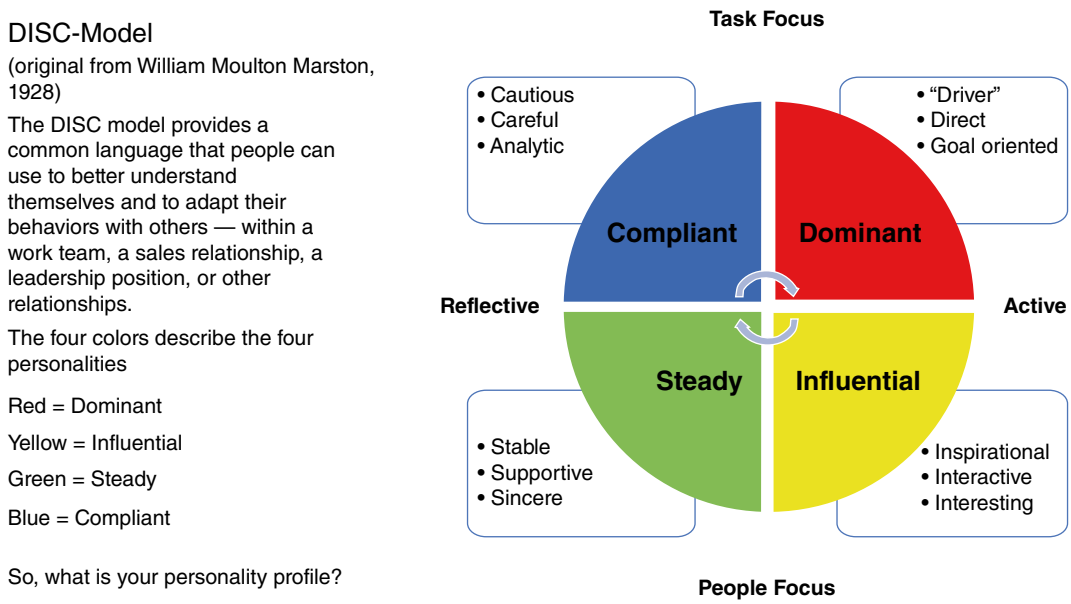


Fig. 27.5 Graphical representation of the DISC model, a personality profile

During periods of separation, it is important for the father to continue playing that particular role. In therapies, we see various kinds of fathers: from uninterested fathers who neither pay alimony nor look after their children, to extremely committed fathers who are kept from being a father to their children by all (legal) means imaginable. The way these separations are handled is also crucial for the mental health of the children.

"Blood Brothers": Friendships Among Men

Most social contacts in families originate from and are maintained by women. Many friendships between men, on the other hand, are based on joint activities such as playing sports, watching football, riding motorcycles, being involved in a club, doing handiwork or attending a cooking course. It is important for contacts to be cultivated continuously and not let them fall victim to family and work.

> "It's always been my dream to ride along Route 66 on a motorcycle. To be honest, I was just working and taking care of my mother and my partner," says a 37-year-old car rental business owner who seeks psychotherapy because of a depressive episode.

Perpetrator or Victim

It is not well known that men are victims of domestic, psychological, or institutional violence (politics, institutions, work, cyberbullying, etc.) just as much as women. More than 90% of prison inmates are male. When it comes to sexualized violence, however, women are more often the victims [38, 39]. Violence is an absolute "no-go," a violation of boundaries, which in turn can lead to violence (brawls, sexual or other offences) or self-harming tendencies (physical illnesses, addictive behavior, a trend toward a conflictual lifestyle). As therapists, we need more courage to address violent experiences of our patients during psychotherapy to better deal with these "traumas."

> When somebody's talking smack, he's gonna get it...

Resist the Onset of Violent Behavior!

Psychotherapy needs to take a clear stance on the issue of violence and needs to better educate the population. Men who have experienced violence often do not talk about it because they are ashamed, and their gender identity does not allow them to do so. We need sensitivity to the topic.

Other reasons for men seeking psychotherapy because of problematic social relationships are:

- Difficult separations.
- Childlessness.
- External relations.
- Detachment issues from parents.
- Feelings of insecurity because of gender identity.
- Unfulfilled sexual desires.
- Financial distress.
- Trouble with the law.
- Diverging values from transcultural, interreligious networks of relationships.

Balance Model: Vision and Mission

Vision + Mission
Hobbies
Inner values
Spirituality
Culture, tradition, rituals
Regulation of virtual world vision

> If you want to build a ship, don't drum up people to collect wood and don't assign them tasks and work, but rather teach them to long for the endless immensity of the sea. (Antoine de Saint Exupéry)

Visions Are Powerful: Visions, Goals, Traditions

We need visions, inner images, goals, to embed ourselves within a larger frame of reference, such as culture, tradition, and a connection to something divine, to lead a fulfilled life [11]. Founders

of religions, literary men, and spiritual-oriented scientists such as Carl Gustav Jung have dealt with the question of what holds all people together, deep on the inside. A well-known concept is Jung's description of a "collective unconscious" as the transpersonal layer of the unconscious, "but this personal layer rests upon a deeper layer, which does not derive from personal experience and is not a personal acquisition" [40]. Jung instead sees it as transcultural, originating from myth, fairy tales, and rituals. The commonality of the human psyche and the accomplishments in a person's life become clear and tangible as soon as the prevalent archetypes are recognized.

As part of a vision quest or vision work, we ask men the following questions as a form of mental exercise:

- What do you *really want* in your life?
- What is your vision right now?
- Don't lose your *passion.*
- Follow your *intuitive power.*
- Remain *curious.*
- Outdated? Discover yourself in a new way and *simplify your life.*
- Do you have hobbies and resources?

In the natural and significant changes in life, there is always a need for a new adaptation of one's own vision and mission in life. A Jewish–American psychologist and psychotherapist of German origin, Erik Erikson, outlined what is most likely the best-known psychosocial phases of self-development in our field. Each change of phase is connected to new questions of meaning, changes of life, competences to act on, social changes, and new questions of identity. Changes need new vision!

Dücker, as shown in Table 27.1, has outlined these phases in a way that is easy to understand and emphasizes the social framework [41].

Leaving Your Comfort Zone: Change

> You never change your life until you step out of your comfort zone; change begins at the end of your comfort zone. (Roy T. Bennet)

Every change and the resulting adaptation of our habits and social companions, requires us to leave our "comfort zone" [42]. We grow with the challenge, bit by bit, like a snake that sheds its skin, only to produce a new one. This process is associated with diverse feelings such as fear, uncertainty, and doubt. Whatever "used to be" is no longer applicable, but "what is now" has not yet been found. It is necessary to embark on unknown terrain and to rely more and more on instincts, one's own attentiveness, and one's new vision. Spiritual anchors, rituals, and the feeling of being connected to something greater can help:

Table 27.1 Phases of change, according to Dücker [41]

Psychosocial "crises"	Relevant people of trust	Psychosocial relations	Elements of social order
Trust vs. mistrust	Mother	Receiving, giving	Nurturing and caring for a child Support
Autonomy vs. shame, doubt	Parents	Withholding, giving	Practicing obedience, self-worth
Initiative vs. guilt	Family	Act, "act as if" (pretending)	Parents and idols as role models
Sense of purpose vs. feelings of inferiority	Neighborhood, school Peer groups	Doing things, creating, questioning	Things and people in the environment
Finding identity vs. diffusion of identity	Peer groups Role models, idols	Who am I?	Opinions, sentiments, ideas, ideologies
Intimacy and solidarity vs. isolation	Friends, sexual partners, rivals	Self-discovery through things that are unfamiliar	Cooperation and competition
Ability to procreate vs. self-isolation	Workplace, household	Creating, caring	Upbringing and tradition
Integrity vs. desperation	All of mankind, divine entities	Accepting self-development and evanescence	Wisdom or inflexibility

- Your comfort zone makes you feel safe, but it can also be a place of stagnation.
- Every process of change makes you uncomfortable.
- The old situation is history, and the new one is not built in 1 day.
- You need good and powerful vision to go through the process.
- You feel uncertain, nervous, sometimes aggressive and depressed or full of hope—this is all normal.
- The new one is not 100% perfect; it needs time to grow.
- Spirituality could give you basic trust, solidarity, and meaning.

A 46-year-old business economist said that whenever his boss accused him of poor work, he would end up in a state of complete helplessness. He has his department under control, works hard and is very popular among employees. The helplessness he experienced was so violent that he was hardly able to react in a sensible manner.

We were able to work out that these situations reminded him of his old mathematics teacher, who, over the course of 2 years, tormented him at school. Even the smell of his office reminds him of the old classroom. In addition, he had feelings of existential insecurity. Using these findings, the patient could change his life: he spoke openly with his boss, reduced his working hours, spent more time with his family and friends, and started viewing his financial means from a realistic standpoint. This process of change was visualized with the help of the balance model, and the important changes in all respective areas of life were highlighted.

Discover and Respect Your Resources

When treating men, it is essential to ask them about their skills, hobbies, and resources to activate positive associations. Fixation on mental issues and symptoms is not helpful in establishing meaningful visions.

At the beginning of resilience training for engineers at a big airline that lasted several months we asked the participants about their hobbies. It was unbelievable how many exciting things were brought to light: one of them built fireplaces, the other had just restored his third vintage car, another ran a cocktail bar parallel to his job, another had cycled through China, and another took care of his paraplegic friend.

The ice was broken and a space for constructive work had been created (even though, originally, we were sent to treat underlying physical stress factors and outplacement developments).

Clinical Applications of a Gender-Sensitive Approach

In addition to the tool of theory-based training in a coaching-oriented methodology, a therapist can use his own personality, including his sexuality and relationship skills, as a tool. Therefore, mindfulness is necessary to work out what the working and transferring relationships that develop based on the gender of the therapist and client are like [43]. We are advocating a gender-sensitive therapeutic approach. Additionally, there seem to be differences between female and male clients when it comes to determining what for, how, and why they are seeking psychotherapy [34]. These differences are displayed in Table 27.2:

What a Man Wants: Special Features of the Therapeutic Setting

So far, there are few studies on the required differences in a therapeutic setting with regard to the treatment of men and women. A Canadian website (www.mantherapy.org [15]) has pushed forward and implemented an innovative online platform as a way of offering low-threshold therapy for men. The aim was to establish a casual setting to encourage men to reflect on themselves to reduce addictions, suicide attempts, and depression. On the website, a fictitious therapist, Dr. Mahagony, sits in a room filled with typically "manly" ornaments (sports certificates, hunting trophies, a large wooden desk) and addresses the men openly and with wit. He treats them at eye level and more as a friend than a therapist. The self-tests are designed humorously and in simple

Table 27.2 Psychological gender differences between men and women

Men	Women
Active coping, "acting out behavior"	More feelings such as: doubt, guilt, depression, fear, grief, worry, anxiety
Aggressive coping mechanisms	More dependent on positive feedback, lower self-confidence
Prone to addiction	Less action-oriented
More suicides	Stronger extraversion, empathy, relationship-orientation, worse differentiation? ("Cost of caring")
Action-oriented	Talk about feelings more often
Less relationship-oriented	Embarrassed because of their own body, social skills, or performance
Lower perception of symptoms of illness	Role shift after motherhood and menopause
More averse to seeking help	Depressive powerlessness, possible acquired helplessness, or feeling of dependency
Do not discuss their feelings as often	
Embarrassing topics are avoided	
Tend to trivialize their situation	
Need recognition	

language. Dr. Mahagony also shows how changes are possible using practical examples and opens himself up.

In a practical setting with men, it is necessary to consider the following points [44]:

- Nonverbal: shaking hands, greeting with a smile, eye contact.
- Formal respect, for example, not being sent away despite appearing late.
- Male attributes within the scope of therapy.
- Clothing and hygiene of the therapist.
- Beverages.
- Administration outside the therapy session.
- Emotional support through self-revelation of the therapist.
- Clearly addressing the relevant topics.
- The therapist does not act like an expert but a coach.
- The therapist also asks about topics that go beyond the problem areas.
- The therapist is mostly chosen by recommendation; often, the client has already gathered information about the therapist by listening to a lecture or looking them up online, "blind dates" are virtually non-existent.
- The client needs to be required to cooperate.
- The therapist teaches client skills.
- The therapist recommends books, movies, and educational material.
- Working with a questionnaire, "homework".
- The therapist recommends an attorney, credit counseling, social services.
- Integrating additional people such as partners, colleagues, superiors, etc.

A quick journey toward self-reflection for our colleagues [12]:

- How do I design my practice in a manner that makes men feel accepted?
- How can I detach myself from gender-specific thought patterns and integrate both male and female role ideals?
- As a therapist, how can I better understand a man and all his various facets?
- How can professionals win the trust of men during psychotherapy?
- How can I understand the man behind the symptoms?
- Which needs are repressed in men?
- What does a man replace his actual needs and desires with?
- How can I understand male courage, male anger, and feelings of revenge, but also feelings of guilt and shame?
- How do I break the chain of powerlessness, anger, aggression, and guilt?
- How can I understand intergenerational tasks that are required of men?
- Wanting female attention while rejecting it at the same time: how can a man escape this dilemma?
- How do I get a man to experiment?
- How do I awake the curious boy that is inside the strong man?
- How do I heal the relationship to their own father/grandfather?
- How can I help the man understand himself as the son of his mother and father while still going his own way?

- How can I involve the partner or other related persons in a productive way?
- What different techniques do I have at my disposal when I am treating men?
- How can I work with groups of men?

Clinical Vignette

A 53-year-old man sought therapy after a recommendation from a general physician and said: "I've been through a lot lately..." With the help of the balance model and extensive anamnesis, we quickly found out about the various obstacles [12] in the patient's life. Figure 27.6 displays the balance model of the respective patient.

By asking questions, the situation and relationship toward his parents became very clear: he had learned that, as a man, he had to "support women" and fulfil his father's athletic hopes that were projected onto him. When we made him create a family tree on his own, the father was above all the other people. The patient knew very little about his own family. One of the central guidelines in his upbringing was to be "hard on oneself," which manifested in a year-long pursuit of the sport of cycling. No one in the family talked to each other and "feelings" were a foreign concept. The patient fulfilled the wishes of the parents whom he idealized and unconsciously prohibited himself from "expanding" and founding a family of his own. The seventh stage according to Erik Erikson, "intimacy vs. isolation," was clearly shifted toward the latter aspect.

The inner desire for peace, security, and emotional attention could only be "earned" by many physical ailments. For him, the hospital was a place of peace and care. At the same time, the hospital was also a place where aggression took place, in the form of anger, which was directed toward the strict parents, the sport, or the medical system (which was manifested by him demanding many surgeries). The patient proceeded to shift the aggression toward the working world by displaying aggressive behavior toward his colleagues and superiors.

Only after a period of time was it possible for the patient to soften his "hard shell." He contacted his sisters, was able to enjoy the sport ("I can cycle with joy now"), and realized that he was very afraid of losing his parents. He sought a conversation with his father, who wanted to learn more about the "lost generation." His girlfriend broke up with him via text message, which made him fall back into a crisis for a short period. After a short time, however, he found a girlfriend on a dating platform. With her, he was able to experience new bonding patterns for the first time and was able to reciprocate!

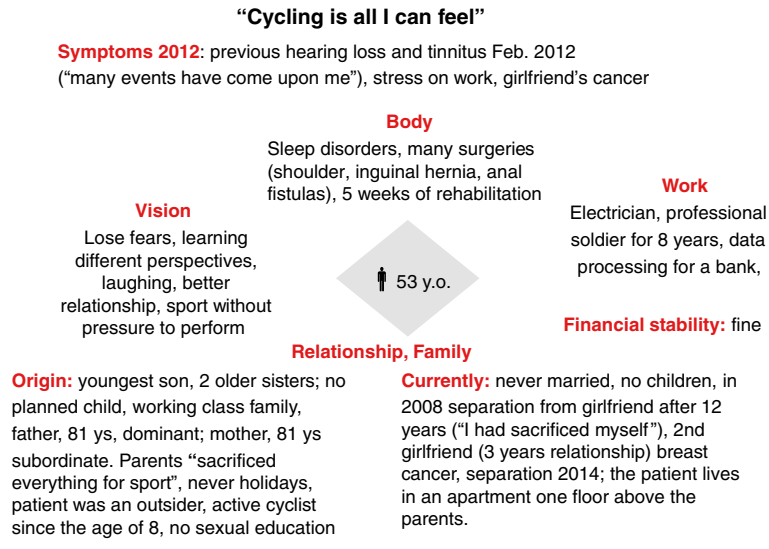


Fig. 27.6 Adapted balance model

Summary

With the help of the balance model of positive psychotherapy, the most important areas of life can be recorded in a structured way and specified in a gender-sensitive way [45]. Men do not need a "different" form of therapy. Therapists only need to be aware of specific "male issues," male relationships, and the male point of view of the symptoms. It is important to meet the men in a low-threshold scenario and act more as a coach than as a superior. The setting should also meet the needs of men and each therapist should practice self-reflection on the individual topics! Men feel more secure when met with comprehensible ideas for action and can then, in the second phase, report more easily on embarrassing and critical topics. Visualization of the topics developed, clear tasks for the time between sessions, resource activation, book recommendations or suggestions for physical activity can set change processes in motion. It can also be helpful to include important persons of reference and to make free use of technical media [14].

Key Points

Poorer physical and mental health compared with women (globally), longer working life, working under dangerous conditions, rare medical check-ups, higher likelihood of addictive behavior, trivialization of symptoms, forced to be "a strong man" by the identity construct (and social obligation), gender-sensitive psychotherapy, balance model of PPT, individual biological, psychological, and social health model, concrete plan of action, health behavior, sexuality, stress regulation, working conditions, family, fatherhood, dealing with violence, developing visions and resources, therapy at eye level, issues relevant to men.

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28 Positive Psychotherapy in Virtual and Remote Settings (Online Therapy)

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Introduction

Although online psychotherapy (also referred to as remote, virtual, telepsychiatry, telepsychotherapy or teletherapy) has existed for a long time, it was COVID-19 that gave a powerful impetus for its development and implementation: thousands of people around the world, being in isolation, needed psychological and psychotherapeutic help that could not be obtained in a traditional offline way.

In recent years, professional unions of psychologists and psychotherapists have conducted a number of studies to analyze this experience [1–4, 9–11]. The report of the European Association of Psychotherapists shows that online psychotherapy has proven to be effective, and during the pandemic there has been a dramatic paradigm shift in favor of online psychotherapy. This was made possible because psychotherapists have successfully adapted to the new conditions, demonstrating flexibility and increased efficiency while adhering to ethical standards. The restrictions caused by the pandemic also contributed to its development. The stigma of seeking psychotherapy has decreased, as the ability to receive help at home has made psychotherapy more accessible [3].

Thanks to online formats, psychotherapy is becoming more inclusive, accessible to people from small towns who do not have access to psychotherapy services, to people with disabilities, and to patients with phobias. Not having to travel to a therapist's office reduces anxiety related to travel arrangements (parking, being late due to traffic delays, and other barriers). Since the end of the pandemic, clients and therapists have appreciated the potential of online therapy to save time, energy, and money on transportation and office rent.

At the same time, the online format has certain limitations. Some clients do not perceive this form of work as effective and do not trust it. In this case, it is very difficult for a psychotherapist to engage them in work.

Systematic studies [1–4, 7–11] describe the general advantages and limitations of online therapy, and professional psychotherapy communities have developed and implemented guidelines for the effective organization of online work alongside ethical standards and guidelines for psychotherapists for organizing online work [1, 2, 6].

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_28

Positive Psychotherapy Perspectives on Online Therapy

From the point of view of the four spheres of life in Positive Psychotherapy (See Chap. 2), the online form of organizing psychotherapy has the most significant differences from the traditional model in the area of "Body": in the organization of physical space of interaction and physical sensations. It is the remote organization of the physical space of psychotherapy via the Internet that leads to changes in the construction of therapeutic relationships (the "Relationships" area), the organization of the therapeutic process and the methods and techniques used (the "Work/Achievement" area), and the very meaning of online therapy (the "Meaning/Values" area).

One of the significant limitations of online psychotherapy is the therapist's inability to fully control the physical space of interaction as safe and confidential. For many clients, it is the psychotherapist's office that becomes the first example of a physical safe space in which they develop a safe therapeutic relationship, gradually internalizing this experience. In online work, the client has to provide a safe space for psychotherapy on their own (to the best of their ability), which is not always possible. Other limitations of online psychotherapy include frequent domestic obstacles that arise during sessions (interference from relatives, distraction of the client by pets, and other sources) [1–4, 9–11].

In our opinion, online psychotherapy redistributes the balance of responsibility between therapist and client, making them more equal partners in communication. While in the traditional model of psychotherapy, the client comes to the therapist's territory, online therapy allows both the therapist and the client to visit each other's physical space and bring their own to it. In his time, by declaring the principle of hope and the principle of counseling and self-help, N. Peseschkian contributed to changing the traditional idea of the dependent passive role of the patient in psychotherapy. In Positive Psychotherapy, the client is seen as a person who, in addition to symptoms and conflicts, has basic and actual capacities and the ability to cope with their difficulties. From this position, the online therapist gets a unique opportunity to see the client in the territory of his or her life in real time, not fantasizing (the "Meaning/Future/Imagination/Intuition" area) about what the client is like outside the office, but clearly observing his or her daily life with his or her own eyes in his or her home, office or even car. (the "Body/Senses" area).

Many clients use the opportunity of an online session to show the therapist their room, children, pets, or favorite things [1, 3]. From the standpoint of Positive Psychotherapy with a humanistic vision of a person, such an acquaintance with the client's whole personality, and not just concentration on the symptom with which he or she came to the therapist's office, is extremely important.

One of the limitations of online work is the lack of opportunities for the client to escape from their everyday life during therapy [1, 3]. At the same time, in our opinion, it is precisely being in this everyday life together with the therapist that can help the client make changes in it here and now. N. Peseschkian proposed the idea of psychotherapy of everyday life as opposed to the Freudian psychopathology of everyday life. Working with a patient who is at home or at work makes it possible to diagnose in real time his or her life balance and spontaneously use the surrounding space and objects for diagnostic and therapeutic purposes. From the perspective of Positive Psychotherapy, the client can start implementing the desired micro-events in his or her daily life right during the session with the therapist, following the principle "if you want to have something you've never had, start doing something you've never done before."

This also applies to another limitation of online therapy – everyday interference, when other people or animals try to contact the client during the session, or the client is distracted by certain issues [1–3, 9–11]. From the standpoint of Positive Psychotherapy, we can consider such cases not only as a manifestation of resistance or other client defenses, but also as an opportunity for the therapist to diagnose the ways in which the client processes actual conflicts, the manifes-

tation of his or her key conflict in relationships with others, the stage of his or her relationship with others, etc.

Changes in physical space in online therapy significantly affect the stages of the therapist-client relationship. While in offline therapy it is the therapist who can manage the stages of the relationship with the client during the meeting (attachment, differentiation, and detachment in PPT, see Chap. 2), in online work detachment can occur prematurely due to technical problems (for example, the therapist's or client's Internet connection fails) or the client can break off contact at any time. Also, as mentioned above, in the process of online therapy, the client may often be distracted by extraneous stimuli, which, on the one hand, can provide the therapist with important information about the client and become the content of further discussion with the client, and on the other hand, interrupts the planned stages of the therapeutic process.

Many of the recognized limitations of online therapy [1–4, 9–11] are related to the difficulty of establishing attachment as a stage in the therapist-client relationship. According to V [12]. Karikash's theory of a two-phase transition in the therapeutic relationship, the first phase of the therapist-client relationship is interpersonal contact, which only later moves to the phase of therapeutic contact with the phenomena of trust, safety, sincerity, congruence then emerging.

Online work limits the opportunities for interpersonal contact between therapist and client as a person to person, and thus establishing contact at the beginning of therapy requires more effort and time. In particular, the lack of the possibility of a holistic physical perception of each other by the therapist and the client (online images do not give an objective idea of a person's height and weight) creates space for imagination and can lead to the formation of a distorted image of another person. Online work does not allow the therapist to see all parts of the client's body (for example, in case of self-harming behavior) or smell the client (including the smell of alcohol or drugs). In general, in online work, it is much more difficult for a psychotherapist to notice changes in the client's breathing and other vegetative manifestations [1–4, 9–11].

In online work, there are difficulties in maintaining eye contact: both a lack of such eye contact (if looking at the screen at the other person's face does not coincide with the angle of one's own video camera) and its excess (looking at the screen automatically implies looking at the other person's face) [3]. In addition, most online communication technologies assume that the client and the therapist can see not only each other, but also their own video. In this case, we can say that there are not two but four objects in contact. Some clients are distracted by their image and experience negative emotions associated with it. Thus, the content of the ''I″-concept becomes the subject of each therapy session, regardless of the client's stated request.

A significant part of the recommendations for organizing online psychotherapy [1, 2, 6] is designed to minimize physical obstacles to establishing relationships at the attachment stage during a therapy session.

On the other hand, psychotherapists testify that for some clients, online contact, in which the emphasis of perception is on each other's faces, makes contact more intimate, which helps to establish trust and sincerity [3].

Positive Psychotherapy as a psychodynamic approach to psychotherapy at the stage of differentiation involves working with the phenomena of transference and countertransference, with the client's childhood experience.

Surveys of psychotherapists [3, 9–11] show that in online work it is more difficult to identify and work with transference and countertransference phenomena, which is important for the psychodynamic approach.

According to psychotherapists' observations, being on their own territory gives many clients a sense of security, and they testify that it is easier for them to open up. A client may unconsciously "come" to a therapy session in pajamas or with a favorite childhood toy, which facilitates the process of working with their early experiences. For other clients, the virtual space of online work, the distance from the psychotherapist also helps to reduce feelings of shame and defense mecha-

nisms, increases the client's disinhibition, which creates opportunities for greater sincerity and openness [1–5, 7, 9–11]. In addition, online work in a familiar environment that does not attract the client's attention with novelty and a variety of sensory stimuli makes it easier and faster to immerse oneself in one's own inner world. We observe the same effect during self-discovery groups online: participants note that it is easier for them to concentrate on themselves, they are less distracted by other participants, their movements and actions.

The phenomena of transference require further research in the context of online therapy. We assume that, unlike the traditional child-parent transference typical of offline psychotherapy, in online psychotherapy it is the Internet network that, having "technical power" over both the client and the therapist, becomes a mother figure, and the therapist and client are equally dependent on it as siblings.

Conducting psychotherapy online places additional demands on the therapist's understanding of the phenomenon of key conflict (See Chap. 35) and the ability to work with it, as well as the ability to maintain a balance of politeness-sincerity, support-confrontation of the client during the meeting.

Working online therapist cannot be sure that other people are not near the client and do not influence the degree of his/her openness, and therefore the therapist cannot guarantee the confidentiality of the meeting [1–3, 9–11]. The behavior of clients working from home can often be reserved rather than sincere (a key conflict).

Physical obstacles to online communication (poor sound and image quality, interruptions, connection issues) also often complicate communication between therapist and client [1–4, 9–11]. In particular, clients whose behavior is dominated by the current ability of "politeness" over "sincerity" may often not tell the therapist that they did not understand something because of physical obstacles.

Online therapy, while having certain limitations on the use of body-oriented techniques, such as art therapy methods, on the other hand, provides additional opportunities to work with the "Body/Senses" area. When a client organizes his or her own space, he or she has the opportunity to learn to listen to the needs of his or her body during the session and satisfy them (which is not possible to the same extent during a session in the office). For example, a polite patient with back pain can learn to simultaneously listen to the needs of his or her body in order to take a comfortable, rather than a "beautiful" position and thus develop his or her sincerity. A new experience in the relationship with the therapist, that "your needs are important" and their satisfaction does not destroy our contact, but rather makes it deeper, becomes important in the psychotherapy of polite clients.

Online therapy can also provide an additional opportunity for the client to observe the psychotherapist as a model in his or her reactions to stressful situations of uncertainty due to the technical difficulties of online work, a manifestation of his or her primary actual capabilities (trust, patience, hope, etc.) and secondary ones (honesty, diligence, etc.).

Finally, on the stage of detachment, it should be noted that online work makes it difficult for both the client and the psychotherapist to switch after the session. In the case of working with emotionally difficult experiences, the client does not have the opportunity to distance himself from this work by physically leaving the psychotherapist's office [1–4, 9–11]. On the contrary, the experience of the session remains present in the client's room even after the session with the psychotherapist is over.

We notice that some clients find it more difficult to withstand the time between sessions without contact with the psychotherapist; in online work, it seems that "the therapist lives in my computer," meaning that one can write or call him at any time, always being at the stage of attachment with him.

It is the issues of ethical behavior at all stages of the relationship, including the stage of detachment, that are central to the Ethical Guidelines for the Organization of Online Psychotherapy [1, 2, 6].

In just a few years, psychotherapists have adapted to the challenges of online work, includ-

ing the delivery of body-oriented interventions. The experience of implementing online Breath-Body-Mind programs [13] shows that online work can be no less effective than offline work if specialists receive special training in online work methods, for example, special training in the ability to notice the client's breathing rhythm online, etc.

Conclusion

The spread of online psychotherapy as never before places demands on the transcultural competence of the psychotherapist, and also stimulates further changes in the psychotherapy paradigm toward a salutogenetic model based on the partnership interaction of the psychotherapist and the client in accordance with the principle of counseling and self-help in the method of Positive Psychotherapy. Further research and discussion are needed on the factors of professional burnout in online work and ways to avoid them, ethical aspects of anonymization/deanonymization of the client in online therapy, protection of the psychotherapist from crimes and violence in the online format, and other emerging challenges in the online world.

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Suicide Intervention from the Perspective of Positive Psychiatry and Positive Psychotherapy

Ali Eryılmaz, Tuğba Sarı, and Erick Messias

Introduction

Since the pioneer work by French sociologist Emile Durkheim, suicide has been studied not only as a clinical and individual act but as a sociologically relevant trend. Durkheim, studying trends in the late 1800s found significant differences across countries in Europe and famously reported that Protestant countries had significantly higher suicide rate compared to catholic nations [9]. Since Durkheim's original work, the trends in suicide have been studied and more recently closely monitored. The usual unit to report suicide rates is per 100,000 in the population and the numbers onward use this unit. In the first twenty years of the twenty-first century, suicide worldwide has decreased from 13.6 per 100,000 to 8.8 or a 35% decrease. This change has been uneven across continents with major decreases in Europe (from 20.7 to 11.1 or −46%) and Asia (from 14.4 to 8.6 or −40%), minor decreases in Oceania (10.7 to 9.8 or −8%) and in Africa (10.1 to 9.6 or −5%) and significant increases in South America (5.9 to 6.6 or +12%) and North America (9 to 11 or +22%). Some of the countries with the largest increases in suicide rate in this period are Georgia (+75%), Mexico (+70%), the Philippines (+64%), Brazil (+48%), South Korea (+42%), and the United States (+41%). Some of the countries with the largest rate decreases since 2000 are: Russia (−63%), Ukraine (−45%), Romania (−36%), and Poland (−25%). Across the globe, men die of suicide at a higher rate compared to women. While in countries like Japan and United States, the suicide rate is similar across age groups, in other places, such as South Korea and Portugal, suicide is more prevalent in older adults [6]. Factors associated with higher suicide risk include trauma [16], presence of mental disorders [34], chronic pain [16], substance use disorders [24], living alone [22], access to lethal means, like firearms [31]. Factors associated with lower suicide risk include attendance to religious service [37], social support and connection [18], and meaning in life [20]. Given the burden of suicide across the globe and these combinations of psychological and psychiatric risk factors, Positive Psychotherapy approaches in suicide prevention have the potential to have a significant public health impact.

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_29

Positive Psychotherapy (PPT after Peseschkian) is a holistic approach that offers various resources and strategies for individuals to cope with internal conflict. In this context, the Positive Psychotherapy balance model examines resources for coping with conflict across four main dimensions: body, achievement, relationship, and fantasy. Body dimensions allow individuals to manage conflicts through physical reactions and perceptions, which can also be effective in dealing with suicidal thoughts. The achievement dimension enhances skills for coping with challenges in work and academic life and internalizing this dimension may reduce the risk of suicide. The relationship dimension reduces feelings of loneliness and isolation by strengthening social connections and support networks. The fantasy dimension provides psychological relief by leveraging individuals' imaginations. This model indicates that individuals with suicidal thoughts tend to utilize the body and fantasy dimensions more frequently and that these strategies offer ways to manage emotional pain. Additionally, the dimensions of Positive Psychotherapy are related to individuals' self-perception, interpersonal relationships, and social connections concerning suicidal thoughts. Individuals with suicidal thoughts have been observed to use lower levels of relational, trust, hope, and love capabilities. Enhancing these capabilities can improve clients' quality of life and reduce suicidal thoughts. Positive Psychotherapy aims to systematically address these capabilities and model dimensions, helping clients manage their internal conflicts in a healthier way and move away from suicidal thoughts. This section discusses and examines the topic of suicide in accordance with the balance model, model dimensions, and capacities.

The Balance Model and Suicide in Positive Psychotherapy

In Positive Psychotherapy, one of the resources for coping with conflict is the human body. According to Peseschkian [25], psychological states are shaped by bodily states. Conflicts should be addressed from both psychological and psychosomatic perspectives. Therefore, examining clients' perceptions of their own bodies and bodily organs is necessary. When a conflict arises, it is important to understand why individuals respond with their hearts or stomachs. This understanding can help clients gain insight into their issues and develop a more positive perspective on the psychotherapy process. Physical responses to conflicts include physical activity, sleep, eating, sexuality, bodily dysfunctions, and psychosomatic reactions.

Positive Psychotherapy identifies various resources for coping with conflict, emphasizing work or academic life, relationships, and the domain of fantasy or intuition. Peseschkian [26] highlights that the achievement dimension involves the internalization of norms guiding behavior, where some individuals cope by immersing themselves in work while others evade achievement demands, potentially leading to issues such as self-esteem problems, excessive stress, and fear of failure. Additionally, relationships are crucial, as Peseschkian [25] notes, encompassing connections with oneself, family, and broader social contexts. Individuals may either withdraw into solitude or seek support from others to manage conflict. Furthermore, the domain of fantasy and intuition, as posited by Peseschkian [25], offers a coping mechanism through dreams and imagination, providing psychological relief by envisioning desired outcomes. These coping styles, influenced by individual differences, offer significant insights into addressing psychological issues and enhancing therapeutic interventions.

Examining suicide in terms of the balance model reveals interesting results. It has been found that individuals with suicidal thoughts use the body and fantasy dimensions more than those without such thoughts [2]. To understand why individuals with suicidal thoughts utilize the body and fantasy dimensions more in their coping strategies, it is necessary to consider the psychological nature of suicide and the internal dynamics experienced by individuals in this pro-

cess. Individuals with suicidal thoughts may use their bodies as a coping mechanism. One of the key techniques in Positive Psychotherapy is positive reframing [25]. At this point, positive reframing can also be applied to the use of the balance model in suicidal thoughts. Firstly, converting psychological pain into physical pain may alleviate emotional burden for some individuals. Physical pain is perceived as more concrete and controllable compared to emotional pain. Thus, experiencing physical pain can help individuals suppress their emotional pain. Secondly, self-harm can help individuals release their emotional intensity. This act provides a sense of temporary relief and control. Additionally, for some individuals, this behavior can be seen as an expression of internal conflict. Thirdly, individuals may attempt to manage their feelings of chaos and loss of control by controlling their bodies.

When examining the positive aspects of using the fantasy dimension, several interesting points emerge. Firstly, fantasy allows individuals to escape unbearable reality. By creating a fictional world, a person can escape from current pain and problems. Secondly, in fantasies, individuals have complete control. This contrasts with the helplessness and loss of control felt in real life. In fantasies, individuals can write their own scenarios, providing temporary relief. Thirdly, fantasies allow individuals to express emotions and thoughts that they cannot articulate safely. This is particularly true for powerful emotions such as shame, guilt, or fear. Fourthly, fantasy involves existential questioning of life. At this point, the individual's worldview, religious beliefs, and cultural values play a role [13]. As a result, individuals with suicidal thoughts use the body and fantasy dimensions to cope with their internal conflicts. While the body dimension provides opportunities to concretize and control emotional pain by converting it into physical pain, the fantasy dimension functions as an escape from reality, a source of control, and a means of self-expression. These strategies can help individuals manage intense and complex emotions in their internal world but do not provide a long-term solution. Based on research findings [2], it is necessary to expand the areas of coping with conflict to assist individuals with suicidal thoughts. In this context, it is important to redistribute the high level of energy given to the body and fantasy dimensions to the relational and achievement dimensions, which are other areas of coping with conflict.

In conclusion, the balance model of Positive Psychotherapy offers various strategies across the dimensions of body, achievement, relationship, and fantasy to reduce suicidal thoughts and help clients develop healthier coping mechanisms. In the body dimension, examining clients' physical responses and body awareness can help them manage their emotional pain by making it more concrete and manageable. In the achievement dimension, developing skills to cope with stress and demands in work or academic life aims to enhance self-esteem and self-worth. In the relationship dimension, strengthening clients' social connections and support networks plays a significant role in reducing feelings of loneliness and suicide risk. In the fantasy dimension, allowing clients to use their imagination and intuition provides psychological relief and enables safer expression of internal conflicts. This holistic approach encourages clients to find healthier ways to cope with the intense and complex emotions they experience in their inner world while moving away from suicidal thoughts (See Table 29.1).

Table 29.1 Balance model and intervention for suicide

Balance model	Activation strategy	Suggestions
Body	Addressing physical reactions and body awareness	Explore the client's perceptions of their body and physical reactions to suicidal crisis. Encourage healthy physical activities and routines. Teach relaxation techniques and stress management. Teach impulse control techniques
Achievement	Enhancing academic and work-related functioning	Examine the client's work or academic performance and related stressors. Promote goal setting and time management skills. Address self-worth and self-esteem issues related to success and failure.
Relationship	Improving interpersonal relationships and support	Explore the client's relationships with significant others and social networks. Encourage healthy communication and conflict resolution. Promote involvement in social and community activities.
Fantasy	Utilizing imagination and creativity for coping	Encourage the use of imagination and creativity as a positive outlet. Help the client develop safe and constructive fantasies. Promote the exploration of dreams and intuitive insights for self-understanding.

The Relationship Between Model Dimensions and Suicide Prevention in Positive Psychotherapy

Positive Psychotherapy is a treatment modality with analytical, humanistic, and intercultural foundations [25]. This model proposes interventions for various mental disorders. A significant feature of Positive Psychotherapy that represents its analytical aspect is the concept of model dimensions. These dimensions relate to the factors influencing the formation of an individual's self and personality. In this context, the "self" dimension pertains to the individual's relationships with their mother, father, and other family members; the "other" dimension relates to the nature of relationships with parents; the "we" dimension involves the family's relationship with the external environment; and the "core we" dimension encompasses the family's values, beliefs, and spirituality [13].

In the psychotherapy process, it is crucial for clients to identify and understand the causes of their problems to take initiative in resolving them [12]. Here, model dimensions play a significant role in understanding the causality of suicidal tendencies. Literature suggests that parental psychopathologies significantly impact the development of suicidal thoughts and tendencies. For instance, parents exhibiting impulsivity, aggression, low cognitive functionality, helplessness, and anxiety are considered significant risk factors for their children's suicidal thoughts and attempts [19]. Pathological parental patterns are reflected in interactions with children [38]. These interactions affect the child's self-acceptance, self-love, self-worth, and ability to learn skills that protect and develop mental health. In pathological situations, however, children may struggle with self-acceptance, self-love, self-worth, and self-care [1]. From an analytical perspective, negative representations of the self-related to parents are evident [15], which is considered the "self" dimension in Positive Psychotherapy. When an individual's parents have emotional disturbances, neuroticism, or depression, this is seen as a significant factor leading to suicidal tendencies in the child [19]. Psychopathology in parents also adversely affects marital relationships,

examined within the "other" dimension in Positive Psychotherapy. Increased conflicts within this dimension are regarded as an important risk factor for suicide [38].

Positive Psychotherapy assesses the family's relationships with the external environment as the "we" dimension. According to Peseschkian [25], a child's relationship with the external environment begins within the family. Through the "we" dimension, family members' relationships with the external environment are established. The "we" dimension also involves learning family communication, distinguishing relatives, friends, and enemies. The roles observed within the family influence social life, and the family has a role as a model. Relationships with broader environments such as relatives, school, and neighborhood can also be protective or risk factors for the mentioned issues [19]. Negative interactions in the "we" dimension are seen as strong factors contributing to suicidal tendencies [13].

Negative interactions within model dimensions impair individuals' abilities related to trust, achievement, order, relationships, and hope. Such situations lead individuals to develop negative perceptions about themselves, their future, and others. Negative experiences also adversely affect individuals' belief abilities, contributing to destructive thoughts and cognitive distortions [19]. Individuals may struggle with their achievement abilities, experiencing inadequacy in problem-solving and a sense of helplessness [13]. These factors collectively lead individuals to perceive their suffering as inevitable, intolerable, and insurmountable [19]. In other words, individuals may become unable to use their hope capacities [13], increasingly turning to suicide as a means to end their suffering and problems.

The most important effect of model dimensions is that individuals who receive adequate and healthy support in these dimensions have higher levels of ego strength. Studies show that higher ego strength makes individuals psychologically resilient, which in turn means they are less likely to experience suicidal thoughts [4, 30]. Empirical evidence also supports that increasing ego strength through model dimensions can help individuals distance themselves from suicidal thoughts. A study comparing individuals with and without suicidal thoughts found that those with suicidal thoughts scored lower on each model dimension, with significant differences between the groups [2].

In Positive Psychotherapy, the practical application of the "Self," "Other," "We," and "Primal we" dimensions involves a systematic and holistic approach to addressing suicidal thoughts. For the "Self" dimension, therapists focus on exploring the client's self-perception and internal conflicts, examining relationships with parents and family members, and promoting self-acceptance and self-esteem. The "Other" dimension emphasizes improving interpersonal relationships and conflict resolution skills, examining the quality of important relationships, and encouraging healthy communication. The "We" dimension addresses the client's social interactions and community connections, promoting participation in social activities and enhancing social skills and support networks. Finally, the "Primal we" dimension integrates the client's values, beliefs, and spirituality into the therapeutic process, exploring these aspects, encouraging involvement in meaningful activities, and helping the client connect with a broader community and develop a sense of purpose. By systematically addressing these dimensions, Positive Psychotherapy promotes a comprehensive approach aimed at reducing suicidal thoughts and helping clients develop healthier coping mechanisms (See Table 29.2).

Table 29.2 Model dimensions and intervention for suicide

Model dimensions	Activation strategy	Suggestions
I	Addressing the client's self-perception and internal conflicts	Explore the client's relationships with their parents and other family members. Help the client develop self-acceptance and self-love. Encourage self-esteem and self-worth.
You	Enhancing the client's relationships and resolving conflicts	Examine the quality of the client's relationships with significant others. Help the client develop conflict resolution skills. Encourage healthy communication and relationship-building.
We	Improving the client's social interactions and community ties	Explore the client's relationships with extended family, friends, and community. Encourage participation in social activities. Help the client develop social skills and support networks.
Primal We	Fostering the client's values, beliefs, and spiritual connections	Explore the client's values, beliefs, and spirituality. Encourage engagement in meaningful activities. Help the client develop a sense of purpose and connection to a larger community.

The Relationship Between Capabilities and Suicide in Positive Psychotherapy

In Positive Psychotherapy, individuals are believed to be born with capacities for knowing and loving. Secondary skills develop as a reflection of the capacity for knowing, while primary skills emerge from the capacity for loving. Research comparing individuals with and without suicidal thoughts has found that those with suicidal thoughts tend to use their relationship, trust, hope, and love skills at lower levels [2]. To understand why individuals with suicidal thoughts use these skills less effectively, it is necessary to examine the psychological functions of these skills and the inner worlds of such individuals.

Negative relationships are a significant risk factor for suicide [19]. Interpersonal conflicts, social isolation, and lack of support can increase feelings of loneliness and hopelessness, which may disrupt mental balance and lead to suicidal thoughts and behaviors [17]. Individuals with suicidal tendencies often have poor social skills. Deficiencies in communication, empathy, and conflict resolution make it difficult to establish and maintain healthy relationships [29], leading to social isolation and loneliness that contribute to suicidal thoughts.

Trust is another crucial factor in protecting against suicide. Low levels of trust can drive individuals toward suicidal thoughts and behaviors [2]. Trust encompasses both self-trust and trust in others. For individuals with suicidal thoughts, past traumas, betrayals, and trust-breaking experiences can undermine their sense of trust. These experiences make it challenging for them to trust others and even themselves [19]. Such individuals may experience paranoia and excessive skepticism, which hinder the formation of trust-based relationships [2].

The relationship between hope and suicide risk has been extensively explored in the literature, with hope considered an important warning sign for individuals at risk of suicide [7]. Hope is seen as a protective factor against suicidal tendencies across various developmental stages [21] and even for prisoners in custody. In Positive Psychotherapy (PPT), hope is viewed as a fundamental skill derived from the capacity for love and plays a key role in the therapeutic process [13]. Individuals experiencing hopelessness often exhibit a lack of motivation to achieve their goals, which can lead to suicidal thoughts. Research indicates that hopelessness may be a stronger predictor of suicide than depression [32]. Hopelessness is one of the most powerful determinants of suicidal thoughts. The scientific rationale for low hope capacity involves individuals'

Table 29.3 Capacities and intervention for suicide

Capacities	Activation strategy	Suggestions
Relationship	Establish a strong therapeutic relationship with the client	Accept the client as they are without judgment. Practice active listening and show empathy. Create a safe and sincere environment. Set healthy boundaries in relationships
Trust	Enhance the client's trust in themselves and others	Highlight the client's past successes and strengths. Set small and achievable goals. Strengthen the therapist-client relationship.
Hope	Strengthen the client's positive expectations for the future	Set attainable and concrete goals. Create positive future scenarios. Emphasize the client's strengths and resources.
Love	Develop the client's capacity to love themselves and form bonds with others	Increase the client's self-awareness. Help the client develop positive thoughts and emotions. Encourage self-care practices.

tendency to view the future with pessimism and helplessness. This perception makes it difficult to develop positive expectations for the future. Continuous experiences of failure, frustration, and lack of control can lead to learned helplessness, weakening one's belief in the ability to change future circumstances and increasing feelings of hopelessness [19]. In PPT, hope involves believing in a way out of pain and stress and having faith in a better future, including setting life goals and a sense of direction [25]. Thus, hope can be a critical factor in treating and preventing suicidal thoughts.

According to Positive Psychotherapy, the capacity for love is one of the most important skills reflecting individuals' ability to love [26]. This capacity includes the ability to love and form bonds both with oneself and with others [13]. Individuals with suicidal thoughts often exhibit low levels of this capacity. They may experience low self-esteem and self-love, making it difficult to show and receive love [19]. These individuals may struggle to form secure emotional bonds, particularly if they experienced attachment issues in early childhood, which can hinder their ability to establish loving and secure relationships in adulthood [2, 13].

In conclusion, the scientific rationale for the lower levels of relationship, trust, hope, and love skills in individuals with suicidal thoughts often relates to past traumas, low self-esteem, cognitive rigidity, and deficiencies in social skills. Improving these skills can enhance individuals' quality of life and reduce suicidal thoughts. From the perspective of Positive Psychotherapy, therapists can use these skills within their relationship with the client. Accepting the client as they are, listening actively, and empathizing can help maintain the client's engagement in the therapeutic relationship. Highlighting the client's past achievements, successes, and sources of pride can demonstrate their capacities and help strengthen their sense of trust. Setting attainable goals, focusing on ways to achieve them, and showing the client their potential to accomplish these goals can activate their hope. Encouraging self-awareness, improving attitudes toward oneself and others, promoting self-care, and fostering positive thoughts and feelings can enhance the client's capacity for love (See Table 29.3).

Research on Effectiveness of Positive Psychotherapy

When we examine the literature, it is seen that research on the effectiveness of positive psychotherapy is carried out both in the form of individual and group therapy. In the context of individual therapy, Peseschkian and Tritt [27] found that the PPT-based therapy process conducted by 37 therapists with 402 patients with different psychiatric symptoms resulted in a significant reduction in the patients' symptoms compared to the control group. Tritt et al. [33] then looked at the long-term scores (up to 5 years) of patients who had participated in the previous study and found that the changes found in the

first study were maintained. Christ et al. [5] examined the efficacy of a PPT-based individual therapy process with 207 clients. This study was based on individual sessions conducted by 21 psychotherapists working at the Wiesbaden Academy of Psychotherapy. The findings revealed that the PPT is an effective method based on the effectiveness factors proposed.

In the context of group therapies, the number of PPT-based studies has increased in recent years. In these studies, positive psychotherapy was found to be effective for a wide range of issues, such as test anxiety [28], psychological resilience [23], well-being [14], effective communication skills (Güler et al. [3]), expanding goals [10], depression symptoms [11], technological addiction [35, 36], and posttraumatic growth [8]. The findings from these studies may suggest that positive psychotherapy is effective in several domains and that these effects have the potential to reduce suicide risk. However, more specific and targeted research is needed to confirm the direct effects of these studies on suicide prevention.

Notably, Bırni and Eryılmaz [3] tested the effectiveness of positive psychotherapy-based group psychotherapy training in reducing depression symptoms and suicide probability in young adults with subthreshold depression symptoms, highlighting the potential of PPT in addressing these critical issues. Subthreshold depression, a condition between mental health and major depression, often progresses to major depression if left unaddressed. Suicidal thoughts significantly increase this risk, emphasizing the need for positive interventions to improve life and hope. Bırni and Eryılmaz used a true experimental design with pretest and posttest control groups to evaluate their intervention. A pilot study involving 36 applicants completing a Google Forms application with the PHQ-9, Beck Depression Inventory and Suicide Probability Scale identified 12 people with severe symptoms who were referred for psychiatric care. The final pilot group of 7 women and 1 man, all of whom had moderate symptoms but did not meet DSM-5 criteria for major depression, showed significant improvements in depression, suicide risk, hope and life engagement following the intervention. This 15-week group psychotherapy program, grounded in PPT principles, involved multiple steps and strategies to help participants understand themselves better, build emotional resilience, and find great meaning. In every session, several steps and strategies from positive psychotherapy were implemented to assist participants in understanding themselves more deeply, enhancing their emotional fortitude, and finding greater meaning in their lives. A brief context of the implemented program is presented in Table 29.4.

Table 29.4 Bırni and Eryılmaz's life-enhancement program

Session	Positive psychotherapy step	Session content
1	Observation and distancing	Introduction of the program, rules, information form, warm-up game, sharing symptoms and problems, session evaluation
2	Observation and distancing	Warm-up game, most severe life event, writing sentences of hope, session evaluation
3	Observation and distancing	Sharing of personal acts of happiness, most severe life event, sharing of sentences of hope, session evaluation
4	Inventory phase	Analysis of problems with behavior control techniques, form filling and sharing in pairs, session evaluation
5	Inventory phase	Behavior control techniques, impact of thoughts on emotions and behaviors, replacing negative thoughts with positive ones, session evaluation
6	Inventory phase/ trustworthiness capability	WIPPTF 2.0 application, explanation of PPT principles, example of reliability capability, practice, session evaluation
7	Situational encouragement and giving hope	Assigning homework, activating the ability to hope, Snyder's theory of hope, writing a hopeful letter, session evaluation

(continued)

Table 29.4 (continued)

Session	Positive psychotherapy step	Session content
8	Verbalization/model dimensions	Summary of homework, analyzing model dimensions, parental relationships, forgiveness activity, writing a human rights letter, session evaluation
9	Verbalization/key conflict	Kindness and honesty balancing, gains and losses, setting a balance motto, practicing balanced behavior, session evaluation
10	Verbalization/core conflict	Exploration of key conflict skills, defense mechanisms, healthy need satisfaction plan, homework, session evaluation
11	Verbalization/punctuality and order capabilities	Punctuality and order ability definition, function, effect, healthy use plan, session evaluation. Homework: to make an organized plan and how punctuality will be ensured within this plan.
12	Verbalization/love capability	Love ability definition, function, impact, self-worth activities, homework, session evaluation. Homework: to read self-worth building activities and self-confidence cards.
13	Verbalization/relationship and trust capabilities	Definition, function, and impact of trust and contact abilities, session evaluation. Homework: to read self-confidence cards daily and to strengthen trust and relationship dynamics.
14	Verbalization/balance model	Introduction of the balance model, in-group dimension analysis, emphasizing the importance of balance, the effort to live a balanced life, session evaluation
15	Expanding goals/preventing future relapses/termination	Preparing a "safety plan" for how to cope with future depressive episodes, general process summary, sharing thoughts and feelings, ending the program

Conclusion and Future Perspectives

Studies have shown that positive psychotherapy (PPT after Peseschkian) is effective in reducing psychiatric symptoms and increasing emotional resilience. It not only reduces symptoms but also helps with the ability to give meaning to their lives, discover their sources of strength, and improve their ability to cope with challenges. All of these are considered suicide-preventive factors. The potential of positive psychotherapy in this area has been demonstrated in a group study specifically aimed at reducing the likelihood of suicide in people with subthreshold symptoms of depression. Positive psychotherapy may be an effective tool for improving quality of life and preventing suicide.

Considering the encouraging outcomes that have been demonstrated by positive psychotherapy in a variety of contexts and demographics, it is suggested that future studies should delve into the potential applications of this technique in diverse cultural settings. Research, such as that conducted by Bırni and Eryılmaz [3], emphasizes the significance of carrying out additional studies, particularly in high-risk situations involving suicide, to fully uncover the potential benefits of positive psychotherapy.

In conclusion, positive psychotherapy provides a broad framework for improving one's mental health and quality of life. The potential for suicide prevention is promising for future research. Ongoing research in this area will not only improve our understanding of true capabilities, but also help to develop more effective interventions aimed at reducing the global mental health problems and suicide.

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30 Features of the Therapeutic Relationship in Positive Child and Adolescent Psychotherapy

Ekaterina Dokunova and Maksim Chekmarev

Background

Interpersonal interaction is the essence and heart of positive psychotherapy (PPT after Peseschkian). The study of the topic of relationships in psychotherapeutic practice in general, and in the field of child psychotherapy in particular, is closely connected with the concept of personality and conceptions of human nature [8]. What is a human being? What drives a human being? What does a person achieve in his development? What are the factors influencing his development, as well as health and illness?

Nossrat Peseschkian noted that the principles of treatment, education, and training have always been determined by the currently dominant concept of a person or an idea of human nature, depending in the broadest sense on the values associated with the life philosophy and the dominant culture at the time" [10].

Consequently, it is the conception of human nature that creates the symbolic field in which the very method of creating a psychotherapeutic environment is constructed, it determines the goals and objectives of psychotherapy, and influences the very possibility of psychotherapy having the potential for development and achieving maturity of the personality. In this regard, we strive for a full-fledged personality-oriented approach. The child's personality is in the process of becoming, this requires consideration by the psychotherapist, and we can distinguish two options for understanding the phenomenon of development by psychotherapeutic schools. The first is reductionist, it proposes to ignore the personal and focus on the behavioral, to consider the child from the point of view of what it is not yet capable of due to its age. For this approach, the demands of society are primary, and the child must be adjusted to its requirements after diagnosis for deviations from the norm. This approach does not see childhood as a special period of psychological life, it is nothing more than pre-life. The second is prospective, it implies that the child's personality is formed at this very moment in the interaction of different realities—the child itself, its family, school, culture, therefore, instead of reduction, it is proposed to expand the attention of the psychotherapist, work with the system of relationships as an ensemble of prerequisites for the formation of personal structures. The child's subjectivity is not ignored, his active participation in upbringing is recognized. As the teacher Konstantin Ushinsky wrote: "The child is not only preparing to live, but it is already living, and this life has its rights and its needs."

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_30

Positive psychotherapy adheres to the second point of view, touching upon both family and culture as integral circumstances of upbringing and development.

From the point of view of counseling and psychotherapy practice, positive psychotherapy can distinguish three points of application of the specialist's activity—***balance, conflicts, and abilities.*** Analyzing them, we can formulate the key differences between the practice of child psychotherapy and work with adults.

The balance of the spheres of a child's life is usually under the control of the parental family. The people raising a child have a decisive influence on the content of the child's life spheres, and this influence is more active the younger the client. As the child gets older, he or she has more freedom of choice, but even for teenagers, significant adults remain a significant factor in choosing education, social circle, possibility of romantic relationships, lifestyle, beliefs, and appearance. This influence varies in different cultures, for example, in traditional cultures, regulation is stronger, they set themselves the goal of actively transmitting socializing knowledge to ensure the continuity of generations, in modernized cultures, parents are more focused on transmitting values than lifestyle, but this still leaves significant adults with a large role in determining the path of life.

The dynamics of conflicts in a child are closely related to the dynamics of conflicts in the family. If the actual and key conflicts are independent for the child in one way or another—he himself is able to form expectations, face reality and choose a reaction to circumstances, then the basic and internal conflicts are mainly formed by clashes of concepts of significant adults, they may not have been appropriated by the child yet, therefore his internal dynamics can be disrupted due to conflicts between significant adults who turn out to be bearers of different values and ways of satisfying needs. Starting from school age, the child begins to actively gain his own experience, to assimilate concepts outside the family from the environment of educational organizations, from peers, small and large groups in which he is a member outside the family. During this period, contradictions between family concepts and concepts acquired by the client from his own experience are frequent. They usually become the content of teenage crises, in which it is important not to lose touch with the parental family, but also to maintain relevance to the new conditions of the constantly changing world. Thus, the child is in a whole system of conflicts, which should be taken into account in the practice of a specialist working with children and adolescents.

The child's actual capabilities are in a continuous process of formation. What is a deficit for an adult is a natural state for a child. It is impossible to demand the development of abilities, you can only create conditions for them. Positive psychotherapy, adhering to the line of formation, approaches the ideas of Lev Vygotsky, who claimed that each quality is born twice—first in the conditions in which it can be manifested, and only then as an internal property. The psychotherapist is involved in the process of developing abilities, along with other significant persons for the child. It can be said that only this aspect is available for work without involving the parental family, but it is also better implemented in alliance with the family system, and this concerns both primary and secondary actual abilities.

Significant differences between child psychotherapy and adult psychotherapy, on the basis of which we can talk about the specifics of child psychotherapy and the construction of therapeutic relationships with the child and his family, are formulated as follows:

Firstly, we are faced with the need to clarify the integral model of family balance, understanding the points of contact between the child and significant adults in the spheres of the body, activity, relationships, and meanings. Speaking about the balance of the client's life spheres, we involve parents in the formation of optimal conditions for his full existence, in which each of the spheres receives support and nourishment.

Secondly, we create a network diagram of conflicts, starting with the child, but then adding to it the characteristics of conflicts arising in the family in connection with the stated request.

Thirdly, we identify the main relevant abilities that need to be developed to resolve the client's problem and create conditions for their development in the relationship with the psychotherapist, but also in the system of family relationships, taking into account the peculiarities of the development of primary (through an example in relationships) and secondary (through gaining one's own experience of regulating life in the learning process).

The Conception of Human Nature and Its Influence on the Different Protagonists

Parent-Child Relationship

The central theory of humanistic psychology is "the idea of a person as an active creator of his own existence, who shows a desire to explore himself and the world around him" [12]. Discovering oneself as a value is one of the most difficult problems of psychotherapist's clients. The fundamental task of child psychotherapy is to return parents to the perception and experience of themselves and the child as values, because the process of development of the personality of not only the child, but also the parents themselves, as well as the child-parent relationship directly depends on how parents perceive their child.

If parents perceive the child as an "empty vessel", they will see their task as "filling" it, offering them their already prepared knowledge and perhaps even a ready-made plan for their life, which probably existed even before the child was born. If a parent adheres to the idea that each of us has a wild, destructive nature hidden in us, their relationship with the child is built on control and correction. If parents look at the child as a "mine of jewels", someone who is already endowed with abilities, believe in the uniqueness of his nature and the value of his own aspirations, then the understanding and experience of their role in these relationships changes, which consists in supporting and creating conditions for the disclosure of the already embedded potential, self-discovery, and development of abilities. Here we can see the influence of the main philosophical concepts formulated in the Modern Age by Thomas Hobbes (man is naturally competitive, aggressive, cares about his own well-being and is guided by it), John Locke (man is a blank slate, and it depends on the environment whether he will turn to good or evil) and Jean-Jacques Rousseau (naturally, a person strives for growth, development, cooperation and love, and damage to nature is more associated with social influence).

In the process of working with parents, we support them in developing their abilities:

- To perceive their child not only as a dependent and closely connected person, but also as a developing and separate personality, with a constantly changing balance of these two states.
- To get involved in the process of knowing their child, gradually giving him the opportunity to act according to his own understanding.
- To experience your parenting as a dynamic state of openness to change and to the actualization of abilities, in which the child's development occurs simultaneously with the development of the parent.
- To meet the idea of the uniqueness of the relationships of unique individuals and direct their attention inside these relationships [6].

The extent to which parents have the capacity to be sensitive and to see their children as separate beings, with their own views, abilities, desires and goals, is related to their own experiences and the circumstances in which their personality developed, and in this regard, individual psychotherapy of one or both parents is often required as a space for exploring one's own childhood experience and the experience of early attachments.

Margaret Rustin noted that even if the initial reason for seeking psychotherapy was the child's difficulties, we again and again discover the basic principles of education: not only do parents educate children, but also the forms of children's

behavior have an educational effect in the opposite direction [6].

Consequently, a special psychotherapeutic target is how parents experience their parental identity, what parents want to convey to their children, and how they do it. The emphasis on the purpose and values of parenting usually returns a positive concept of oneself. The emphasis on the means of educational interaction helps to discover how congruent the goals are with the means and how to create conditions for a consistent parental message.

Therapist-Child Relationship

"Although it is now customary to say that the last thing left for us to know is the cosmos, in fact, perhaps the last thing left for us to know is childhood." Not all adult participants in society succeed in perceiving a child as a unique individual, possessing intuitive wisdom, an innate tendency to grow, the ability to manage their life and creatively interact with their own world [1].

According to Rogers [13], "If I can provide a certain type of relationship, the other person will discover in himself the capacity to use that relationship for growth and change, and personality development will then occur." This attitude can be described as therapeutic; it is a function of the basic attitudes of the child therapist, who is child-centered and wants to know the child and be known by him in the process of developing the relationship.

The therapist's ability to develop a relationship of trust and an atmosphere of emotional safety for the child is not the result of learning a theory or techniques. It is related to the process of self-awareness and self-acceptance. This means that the therapist is well oriented in the space of his own personality, the experience of his own childhood, parenthood, and understands his motives, needs, and values. He is able to show courage to meet anxiety, uncertainty, confusion, despair, and remain genuine and authentic in the relationship, maintaining the living nature of the therapeutic relationship. The relationship should be natural and follow the main stages of development outlined by Nossrat Peseschkian—attachment, differentiation, and separation. In child development, these three stages can be designated by three key words—instead, together and next to. We do not expect from the child an already formed ability to satisfy needs, realize values, and solve problems. They develop together with actual capacities.

Seeing a child in a dynamic perspective presupposes at the first stage the ability of the parent and the psychotherapist to recognize unmet needs (primary capacities are present at the beginning of their development as needs, and secondary ones are necessary external conditions for supporting development and socialization) and to satisfy them for the child. This is the meaning of the word "instead"—at this stage, the development of capabilities occurs when an adult satisfies the child's needs at the expense of his own capabilities, thereby demonstrating an example that can be perceived in a satisfactory attachment relationship. The adult seems to discover the child's inner world, gets to know it, introduces it to the child to it, and expands his inner capabilities. The differentiation stage in the aspect of development presupposes the principle of "together". The child begins to make attempts to independently manifest capabilities, and the adult helps them move to a new level, and accept the child's tendencies to manifest themselves in their own way. Here we help the child to know himself through another, in particular, an adult. The separation stage is the support of the child's autonomy. We are there and ready to validate his methods of adaptation and transformation of reality, to give him confidence in his powers, to promote his innate motivation to grow up. The view of a significant adult who is able to accept the will of the child, his otherness, is important. At this stage, we support the independent development of the client's capabilities. Therefore, the conception of a child in positive psychotherapy is similar to the image of a plant that develops from a seed, initially produces vulnerable shoots, but then is able to exist in an environment alongside others, cooperating with them on the principles of respect for gradually strengthening independence.

Therapist-Parent Relationship

If you ask child psychotherapists about the areas of greatest difficulty in their work, the difficulties of building relationships with parents will be one of the first to sound. The question of how to remain a therapist for a child and at the same time, with the help of relationships with parents, weave a sophisticated pattern of interaction [1] with all participants in the psychotherapeutic process, while experiencing the whole spectrum of feelings (from anger to sympathy) - remains one of the most difficult.

Although child psychotherapy is aimed at resolving or preventing psychological difficulties in children, a cooperative relationship with parents is an important condition for the effectiveness of the psychotherapeutic process, since "they are the bearers of self-help" (family) and "the therapeutic agent of change" [1]. The main categories of these difficulties are related to the following topics: what form of work with parents is possible, what to do if parents do not want to participate in therapy, how to withstand control and intrusion of parents into therapy, idealization or detachment and shifting all responsibility to the therapist, how to withstand complex countertransference reactions and not unite with the child against the parents, how to balance between the right of parents to receive information about their child and the child's right to privacy and confidentiality [1].

A view on the human nature helps a child psychotherapist rely on the idea that behind the facade of any "strange and alien" [2] behavior of parents towards the therapist or the child, there are other unconscious feelings and needs. This means that we consider any human behavior as meaningful, taking into account both transcultural conditions and those conditions in a person's life that fill their behavior with meaning. A person consists of abilities and is driven by the realization of their innate potential for love and knowledge. And although the development of these abilities can be significantly limited or damaged, the meaning of psychotherapy is to "return a person to the original state of health."

First of all, recognizing that they themselves and their child need help is a very subtle and difficult task for most parents. Parents may experience anger, fatigue, guilt, and irritation. It is important for the therapist to be sensitive, because most parents need great courage to face their own vulnerability, to be able to show trust in the therapist, their child, themselves, and the psychotherapeutic process. The psychotherapist becomes an example for parents through their attitude, and how they can relate to themselves.

Parents may experience intense anxiety that the child will change, or that their relationship with the child will change, or that the relationship with the therapist will become very important to the child. They experience fear of losing control, authority, contact with the child, guilt, and shame associated with a sense of failure as a parent, reluctance to disclose very personal information about family relationships, jealousy, envy, and anger. The therapist's reactions may be inappropriate, he may not notice the parents' hurt pride and become entangled in countertransference feelings (Kohrman et al. [14]).

We also often deal with stories told by both the child and the parents themselves about emotional coldness and detachment, lack of proper care and adequate response to the child's needs, or even cruelty, which of course gives rise to complex countertransference reactions that are incredibly difficult for the therapist to manage. Often this is due to the fact that parents really do not know how to react and help their child cope with difficulties and feelings, as well as how to help themselves cope with feelings.

Therefore, developing relationships with parents assumes that we create conditions, such a trusting atmosphere in which it becomes possible to openly discuss feelings, conflicts, and needs that may not be recognized. Thus, developing the ability to see needs and values, potential and prospects for future development behind behavior becomes an important psychotherapeutic task. This ability is transferred by the therapist to both the parents and the child. The methods of therapeutic work with parents vary significantly, and this will require considerable flexibility from

the therapist. The importance of the parent-child relationship must be constantly taken into account by the therapist, and he must treat this complex relationship with deep respect and look for possible ways to support parents in their relationship with the child.

Parenting Concepts

One of the important tasks of child psychotherapy is to establish or restore a normal communication process within the family (between the parental couple, between parents and the child, between the child and the social environment) and often includes rethinking one's own parenting, exploring the concepts of motherhood, fatherhood and childhood. The way parents evaluate and experience themselves in their parenting "significantly affects their interaction with children, and accordingly, the development of children" [1] (Swick and Grawes [15]).

The tool of positive psychotherapy—4 dimensions of relationships (I, You, We, Primary We)—allows us to see how ideas and beliefs about parenting and childhood are significant not only in relationships with a child, but throughout life [9].

Let's give them a description:

I—What qualities are valuable for a parent, and which are unacceptable, for example: a parent should be fair, patient, accepting, strict, demanding, controlling, etc. "A good mother is never angry with her children" "Children must obey their father unconditionally".

You—Here we explore ideas about how parenting affects partnership: how parental roles are distributed between partners, how parenting changes partnership, what ways of interaction in the family exist between parents and children. "Responsibility for upbringing lies with the mother." "Children should not see their parents conflicts" "For the sake of the children, it is necessary to preserve the marriage".

Primary We—In this area, ideas about how parenting affects relationships with other people and groups of people, and how society perceives the image of a parent. "Grandparents are important participants in raising children". "With the appearance of children, a parent must give up social ties and devote himself to the children". "Children do not have the right to decide anything on their own while they depend on their parents".

Primal-We—includes ideas about parenting related to the meaning of life, religion, and the mission of parenting. For example: "Children are the meaning of life, the continuation of the family line", "Parents and ancestors require a respectful attitude" and "If I devote myself to children, in return they must observe the commandment about the need to take care of me in old age".

Most of such concepts were formed in the past, but continue to operate in the relationship between parents and children, causing conflicts and frustrations. "A child is often treated in the same way as the parents themselves were treated in their time, without taking into account the demands that modern life places on the child. Thus, most human conflicts can be seen as a violation of the time factor, that is, as an insufficient integration of the past, present, and future." Using the model of four dimensions of relationships in the psychotherapeutic process, we can not only explore the attitude to parenting now, but also turn to the past (role model)—to the first models of parenting and other factors that influenced the formation of these beliefs, and also consider the future development of these relationships through the tool of 4 dimensions of responsibility. The parent's awareness of their own concepts, which usually create the basis for conflicts, becomes the task of the first stage of therapy, and in the second, these concepts should be expanded, differentiated, become an environment for the development, and not the suppression of the subjectivity of parents and children. Education requires awareness, and not automation, which inevitably occurs if the concepts of parenting remain drivers of the unconscious.

Transference and Countertransference in Child Psychotherapy

Otto Kernberg [16] argued that an expanded approach to countertransference allows a child psychotherapist to see the patient, his parents,

and the therapeutic situation in the most complete form. By countertransference we mean the therapist's perception of all his own bodily sensations, impulses, emotions, fantasies, and thoughts arising in contact with the client [4].

When considering the factors influencing a certain nature of countertransference in child psychotherapy, we obtain the following categories:

1. Emanating from the child in therapy.
2. Emanating from the child's parents and family.
3. Emanating from the therapist himself.
4. Emanating from the child's other social environment and educational and upbringing systems [5].

The versatility of the processes occurring in the psychotherapist requires observation and structuring of countertransference reactions, for which we use the concept of four ways of knowing reality (Balance Model), formulated by N. Peseschkian [3]. According to this concept, there are four channels of knowledge of the world:

– Through sensations (emotions, feelings, bodily sensations).
– Through logic and analysis (thoughts, assessments, impulses).
– Through traditions and personal experience (associations, personal experience).
– Through intuition (fantasies, dreams) [7].

Since in the field of child psychotherapy we often use all types of play therapy and metaphorical objects, the processes of transference and countertransference are manifested not only in emotional and behavioral reactions, but also in the created images, plots, and results of creative activity. Here transference and countertransference can be studied in three aspects: client-metaphorical object, client-therapist, therapist-metaphorical object.

Approximate questions for structuring countertransference processes can be designated as follows:

Bodily sensations and emotions: How do I feel with the client? What happens to my body and sensations and emotions during communication with the client, in the process of observing the game or participating in it?

Cognitive experiences and impulses: What do I want to do and what impulses are born in me in relation to the client or the images he creates? What do I think and say to myself?

Experience and traditions: What does this situation remind me of? Why did I remember this particular story?

Fantasies and expectations: What do I fear or expect from the client? What would I like from him? What do I fantasize about?

It is easy to notice that positive psychotherapy has long been touching upon the transcultural aspect of countertransference, which is associated with the analysis of experience and traditions. Representatives of various psychotherapeutic approaches today also increasingly discuss the concepts of "cultural competence", "cultural sensitivity", "culturally conditioned trauma", "cultural blind spot", and "cultural countertransference" [17–20].

Positive psychotherapy considers the interaction of therapist and client, the therapeutic process and the dynamics of transference and countertransference as a space for exploring cultural values, culturally meaningful interventions that promote the development of intercultural awareness, creating conditions for the implementation of the principle of the uniqueness of the individual, emphasizing the uniqueness of the interactions of the therapist and client, as bearers of a unique cultural experience.

The principle of transcultural sensitivity reflects the importance of interaction from an intercultural position and the study of the questions: What do we have in common? And how are we different? It also considers conflicts not as a clash of personalities, but as a clash of cultural values, including the values of generations. In the psychotherapeutic process, we explore the psychodynamics of relationships in the following directions: intrapsychic, interpersonal, group, and systemic.

Dynamic processes, accordingly, also occur at several levels:

(a) Psychodynamics in the narrow sense, i.e., intrapersonal organization of the conflict flow.
(b) Sociodynamics, i.e., mutual processes in which individuals are involved.
(c) Sociogenesis, i.e., development of a group tradition, which is under the variable influence of the impulses of the group members [11].

Analysis of the content of transference and countertransference in PPT includes the dynamics and content of conflicts, the model of relationships, and specific characteristics of the personalities of the client and therapist (actual capacities) [4]. For example, in the process of interaction with a child and family, we may hear about recurring central themes of conflicts. Discovering the significance of actual capacities and values in family culture (obedience, accuracy, loyalty, thrift, diligence) and their role in the content of upbringing, interpersonal conflicts, in relationship patterns, "is the tip of the guiding thread that we then follow to discover intrapsychic conflicts and their dynamics."

The next step in the psychotherapeutic process can be a study of the experience, traditions and circumstances under which these capabilities acquired such significance in the life of parents, the child and the life of the therapist. With what attitude and needs (primary capabilities) in this experience is the expectation of behavior (secondary abilities) associated. Such an analysis of transference and countertransference, observation of the relationship between parents and children turns out to be the basis of psychotherapeutic diagnostics. The therapeutic process is aimed at creating conditions for the identification and verbalization of their conflicts by the participants, sorting out ways to resolve them through expanding the patterns of relationships aimed at satisfying frustrated needs.

Conclusions

Psychotherapy of children's clients, more than any other, requires attention to the process of interaction, if only because here we are dealing with the psyche, which both grows and heals in relationships. The therapy itself becomes an additional force for education and development. In adult therapy, we deal with consequences, and in children's therapy—directly with the conditions of personality development. This expands the focus of attention, requiring including the family in the process, and promoting the healing of parental concepts.

Cooperation is possible only on the basis of the humanistic concept of childhood, because only it recognizes the intrinsic value of childhood, and invites us to see the child in a dynamic development perspective. This must be observed in the logic of developing current abilities and improving the conceptual environment in which the client grows. Consequently, the therapy itself is not technical, but natural, always containing a component of development and formation, and not suppression and correction.

This approach requires special attention to the processes of transference and countertransference, which reflects the entire family system. Observation by the psychotherapist of his internal process helps to more reliably identify the deficit and frustrated needs brought by the child into therapy. Working with them, we work on the development of the personality through the creation of conditions for a healthy passage through the stage of interaction and development of relationships. Thus, positive psychotherapy in working with children goes beyond simple problem solving; it is important for the therapist to have a vision that influences the psychotherapeutic process and leads to a change in the points of view of all participants - the therapist and the family.

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31 Trauma Counseling and Crisis Interventions Based on Positive Psychotherapy

Tetiana Zhumatii

Introduction

In light of recent events, the problem of psychological trauma is becoming extremely relevant for modern society. There are many events with traumatic potential in the world, such as wars, pandemics, natural disasters, catastrophes, rape, captivity, deaths due to accidents and murder, etc. Although psychological work with psychological trauma has been studied for a long time, the concept of crisis and traumatic situations is not limited to the objective features given by modern definitions of psychological trauma. Each person experiencing psychological trauma needs an individual approach. It is necessary to take into account such psychological factors as perception, understanding, reaction, and behavior of each individual in such situations. Therefore, there is a growing need in society to provide quality psychological assistance to survivors of extreme situations in the form of crisis and trauma counseling. Such psychological assistance is necessary to prevent disorders of human behavior and functioning in the world, as well as to prevent PTSD, adjustment disorders, neurotic and psychotic disorders, and psychosomatic diseases. In this section, we will consider how Peseschkian's positive psychotherapy can be used in crisis and trauma counseling.

Trauma counseling is a short-term psychological aid that lasts from one to fifteen sessions. As a crisis intervention in the first psychological aid, the duration of the consultation can be 20–30 min.

Positive psychotherapy (PPT after Peseschkian) is a short-term integrative transcultural humanistic and psychodynamic resource-oriented method with a holistic vision of a person as a unity of body, mind, and spirit. Therefore, its use in crisis and trauma counseling is highly effective.

The Phenomenon of Psychological Trauma

The concept of psychological trauma emerged within the theory of post-traumatic stress disorder in the late 1980s. This concept has become commonly used in the field of psychology and psychotherapy. The concepts of "psychological trauma" and "mental trauma"are often perceived and described in the literature as synonyms, so we will refer to these concepts in general as psychotrauma. Speaking about trauma in positive psychotherapy, Arno Remmers notes that this term can often be understood from three perspectives: (1) as a potentially traumatic or triggering event, (2) as immediate reaction of the affected

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_31

person, and (3) as any consequences of trauma that may remain after the incident [18].

The American Psychiatric Association's Diagnostic and Statistical Manual of Mental Disorders (DSM-5) defines traumatic events as exposure to the death or threat to life (own or others), serious bodily injury, or sexual violence in one (or more) of the following ways: (1) direct experience of the traumatic event(s); (2) witnessing events that happened to others; (3) learning that the traumatic event(s) happened to a family member or close friend—in case of death or threat to life of a family member or friend, if the event(s) was violent and unpredictable; (4) experiencing repeated or excessive exposure to unbearable details of the traumatic event(s) (e.g., people in helping professions who have constant contact with details of traumatic events) [1].

Judith Herman notes that traumatic reactions occur when the action taken to overcome a situation does not produce the desired result. When neither a resistance nor an escape strategy is possible, the human self-defense system fails and becomes disorganized [4]. Bessel van der Kolk says that trauma is not just an event that happened in the past, but it is also an imprint on the mind, brain, and entire body that these experiences have left. And this imprint affects a person's ability to survive in the present. Y. Medynska and S. Yakushyk [9] define psychological trauma as a reactive formation in the psyche of an individual caused by an intense, emotionally charged, negative, personally significant experience that exceeded the resource potential of mental defense mechanisms and had a powerful destructive impact on human life and functioning. As we can see, there is no complete systematic definition of psychological trauma. Many authors, studying the phenomenon of psychotrauma, describe their conceptual and clinical discoveries. However, each person in a potentially traumatic situation has his or her own individual reactions of perception, understanding, emotional response, and actions, so we can foresee a constant expansion of the criteria for defining psychological trauma, taking into account the transcultural experience of each person.

If we summarize the above definitions and our experience of working with patients who have experienced traumatic events, we can offer the following definition of psychological trauma: *Psychological trauma is a condition caused by the survival response of the psyche, brain and whole body in response to an extremely stressful event that, due to a real or imagined threat to life or physical integrity, or through close contact with violence and death, or due to a report of an unexpected loss of a loved one or destruction of a home/community, overloads a person's coping strategies and exceeds the adaptive resource potential of psychological defense mechanisms, disrupts/destroys the sense of existential security, and has a negative impact on human functioning; manifests itself in feelings of intense fear of the threat of destruction, helplessness, loss of control and loneliness and disrupts a person's ability to function in everyday life.*

Nossrat Peseschkian divided psychological trauma into macrotrauma and microtrauma. He defined a macrotrauma as a significant event in a person's life that requires them to demonstrate various abilities and personal qualities that contribute to adaptation to changing conditions, considering it as a motivating factor for positive changes in a person's life. It is interesting that Peseschkian believed that among psychological traumas there can be not only negatively assessed life events, but also those that are perceived as joyful. Microtraumas are the repetition of many small psychological traumas, which leads to micro stresses resulting from them. These microtraumas have a conflict potential in the form of actual capacities. [17]. Remmers notes that "it is conceptually clearer to talk about traumatic experience, traumatic processing, and the subsequent consequences of trauma" (PPT Handbook 2022) [18].

The American Psychiatric Association's Diagnostic and Statistical Manual of Mental Disorders (DSM-5) defines acute stress disorder (ASD, 43.0) and post-traumatic stress disorder (PTSD, 43.10) as possible processing and consequences of traumatic experiences. For diagnosis, the DSM-5 offers a number of criteria and symptoms that are similar for both disorders but differ in duration: (1) intrusive symptoms that are related to the traumatic event and occur after it; (2) persistent avoidance of stimuli related to the

traumatic event and occur after it; (3) negative changes in cognition related to the traumatic event and occur after it; (4) marked changes in excitability and reactivity related to the traumatic event and occur after it. For ASD, the manifestation of these symptoms is characterized by 3 days to 1 month after the traumatic event, and for PTSD, the disorder lasts more than 1 month, but may be delayed.

As we can see, the phenomenon of stress underlies the processing options and consequences of traumatic experiences, so we need to understand the biological mechanism of stress. N. Peseschkian defined the reaction to stress from a physical point of view as a finely processed message that is delivered by nerve impulses along with hormones to every part of the body. In response to a real threat, in order to survive, an organism has the reactions of fight, flight, and freezing. Hans Selye distinguished between three reactions to stress: anxiety (mobilization), resistance (adaptation), and exhaustion (recovery). [16]: The first phase of stress—the anxiety phase—fuels the body with additional energy that a person should use to resolve the situation (the "Fight" response) or escape from it (the "Flight" response). At the same time, the body is undergoing an adaptation process, which is the second phase of stress, in which the adrenal glands release the hormone cortisol. Cortisol is able to keep a person active longer to combat stress. It has an anti-inflammatory effect and is used to overcome a possible injury. The third phase, exhaustion, occurs when stressors remain in place for too long. Stress reactions last a long period of time and negatively affect the body, as the state of internal mobilization is unreasonably long. The prolonged presence of cortisol in the human bloodstream prevents the body from returning to normal functioning: the immune system is weakened, and diseases develop. Also, high cortisol levels can have destructive atrophic effects on the hippocampus. The connection with long-term memory is impaired. The amygdala, also called the "panic button," becomes overly sensitive and excitable due to prolonged exposure to cortisol, and can quickly trigger fight-or-flight reactions [2]. This stress mechanism is used by the body in any stressful situation that is perceived as threatening to survival. The intensity of the reactions will depend on the strength of the stimulus, the duration of its action, and the state of the organism under stress. Situations that are extraordinary for perception and reaction can become extreme stress.

Types of Traumas

Let's take a look at the types of injuries depending on the intensity of the impact on a person. Historically, psychological trauma was defined as one or more significant macro-events that occurred to an adult. Modern studies of psychotrauma call them traumas of the first type as ones which occur as a consequence of a single unexpected event [10]. However, more and more authors [4, 22] talk about the influence of early childhood through a series of microtraumas on the development of the patient's personality structure and his or her way of reacting in difficult conditions. Some authors call such psychological traumas the second type of trauma (as a result of multiple, repetitive, or prolonged traumatization) [10]. We propose to consider what types of trauma can be found at three levels of diagnosis and symptom development in positive psychotherapy (Iceberg model), where conscious (*level of symptom* and its function) and unconscious levels (*level of conflict* psychodynamics and *level of personality structure*) are distinguished [17]. This will help to form a holistic view of the patient's traumatic experience throughout life by building a trauma map (Fig. 31.1).

At the first level, the tip of the "iceberg" that is observed and realized, we can immediately see one macro-event (monotrauma) or several different macro-events (multitrauma) that will have localization and specific reactions in the areas of the model of the balance of the four qualities of life. These are macro-events that meet the criteria for diagnosing a traumatic event that could have occurred in the present or past, but remained unprocessed by the human psyche. At the second level of diagnosis, the level of conflict psychody-

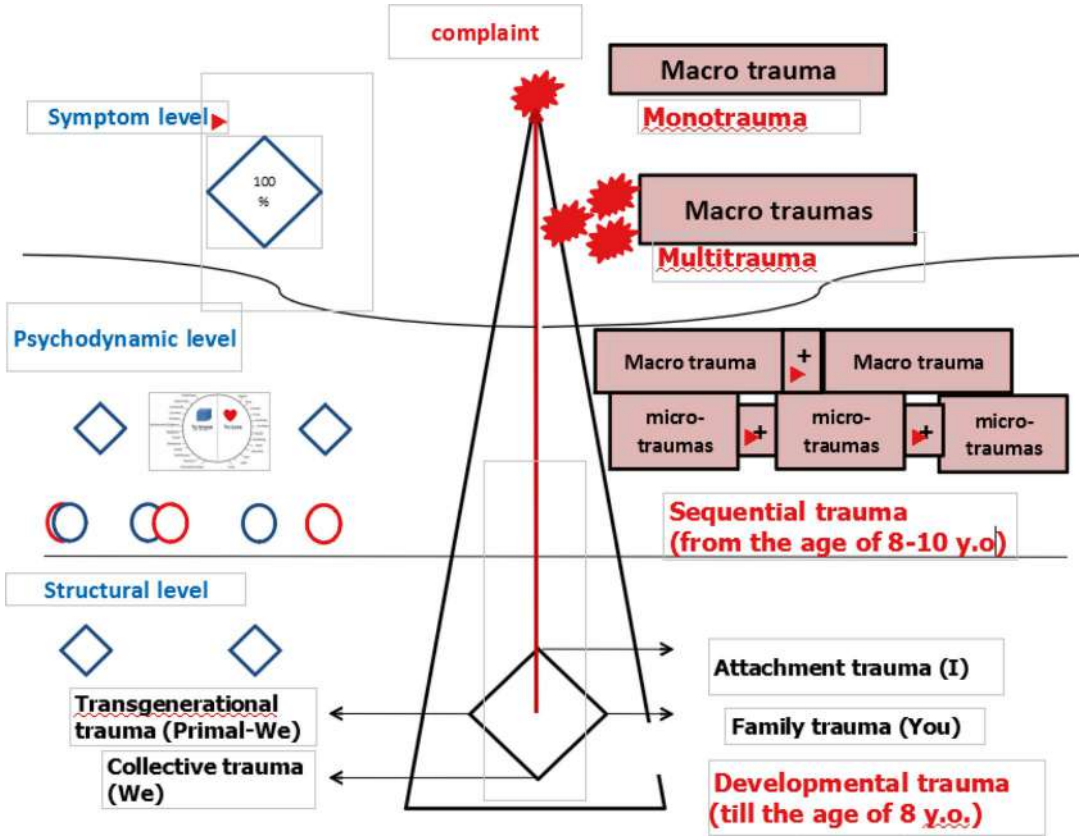


Fig. 31.1 Building a trauma map

namics, which may be unconscious, we can see the impact of a type of macro-trauma or micro-trauma that has been consistently or regularly repeated since a certain period of life. Some authors refer to this phenomenon as sequential trauma, which refers to several or many identical events in a person's life that began between the eighth and tenth year of life [17]. We believe that this type of psychotrauma, due to its consistent repetition, requires the development of certain actual capacities as a way to survive in difficult life conditions while simultaneously depriving certain primary needs and not being able to satisfy them through secondary actual capacities. At the level of the personality structure, which can also be unconscious, let's consider the childhood experience with the help of a model dimensions that demonstrate the mechanism of development of the human ability to experience and express emotional attitudes towards oneself, other people and the world through four types of basic relationships with I, YOU, WE, Primal WE [17]. At this level, we will talk about developmental trauma, which refers to repeated similar or the same macro-traumatic or micro-traumatic events that began before the eighth developmental year of life. [10]. At the level of structure, we can trace the formation of basic concepts and coping mechanisms that may be the content of a basic conflict caused by the traumatic experience of both the patient (the I sphere) and his or her parents, siblings, immediate environment, society (the You, We sphere), and even ancestors and family (the Primal-We sphere). This negative experience can be formed precisely through micro-events that may not directly threaten the child's life, but can be perceived as such.

We propose to consider in more detail the areas of the model dimensions as a potential localization of the types of traumas received in

childhood or inherited, which affect the formation of the personality structure and the ability to cope with potentially traumatic/triggering situations.

"I" Dimension

Attachment trauma (abandonment trauma, rejection trauma, deprivation trauma [21]) is determined by physical and emotional contact and relationships with the mother or the closest caregivers. In positive psychotherapy, there are three leading relevant capacities that are important in the first years of life: patience, time, and modeling. Otto Rank introduced the concept of "birth trauma"—unconscious experience of a person associated with his or her birth [21]. N. Peseschkian also noted that the development of actual capacities begins in the intrauterine period. Therefore, the formation of attachment trauma can be considered from the very beginning of human development through the stages of interaction in positive psychotherapy and the development of actual capacities [23]. A key factor in healthy adaptation to stressful situations throughout life is the so-called internal locus of control, which is formed as a result of attachment, which causes a sense of security and the development of self-regulation skills. Children who have experienced neglect and abuse learn to give up when faced with difficult situations. Childhood experiences can literally affect the biology of the human brain [22].

"You" Dimension

For the harmonious development of a personality, the example of the parents' partnership and the climate in the family is important. Almost all early experiences and negative childhood experiences (poverty, dysfunctional family, divorce, physical violence between parents, parental addictions, etc.) affect a person's ability to survive and achieve well-being. The impact of stressors cannot be assessed from the perspective of an adult. They must be considered in the light of the child's developmental stage and personality. These potentially traumatic events may have different effects on different children, but they should be given enough attention to understand their role in the development of personality structure and the ability to cope with excessively stressful situations.

"We" Dimension

Collective trauma is a psychological trauma suffered by a group of people of any size, up to the whole society, as a result of a man-made or environmental disaster or criminal actions of political or other social actors. Trauma occurs as a result of the process of destruction or irreversible changes due to the living of traumatic experiences in the social and psychological structures of the group, such as values, identities, senses, culture, history (memory), etc. A distinctive feature of collective trauma is not the number of victims, but the fact that in addition to the participants, it affects people not directly involved in the traumatic events, sometimes the whole society [6].

"Primal We" Dimension

Transgenerational trauma is a collective trauma that is transmitted from the first generation of trauma survivors to the second and subsequent generations of descendants through the complex mechanisms of post-traumatic stress disorder [6]. O. Lytvynenko and O. Tereshchenko define transgenerational trauma as a manifestation of emotional and behavioral reactions to real and potential threats in the here and now. O. Tereshchenko defines transgenerational trauma as a manifestation of emotional and behavioral reactions to a real or potential threat in the here and now in people whose experience is not related to living through traumatic situations, but is very similar to living through trauma in their ancestors. Thus, the concepts of survival can be transmitted from generation to generation. O. Lytvynenko and O. Tereshchenko propose to

call this a transgenerational basic conflict [8]. When working with traumatic experience, it is important to understand that an unprocessed negative macro-event or trauma caused by the accumulation of micro-traumatic situations can be reactivated by current traumatic experience and therefore produce a cumulative effect of reactions. Thus, having a comprehensive view of the patient's trauma map, you can better understand the patient's internal conflict dynamics, take into account the peculiarities of the personality structure, and choose a work strategy that is right for him or her.

The Role of Loss and Grief in Traumatic Experiences Processing

When a person experiences trauma, much attention is paid to the experience that caused deep emotional pain. The word trauma itself means "wound, damage". Our experience of working with trauma has shown that traumatic experiences often cause losses and grief. We define loss as the process of a person living through the experience of the death of a loved one. But loss can also be defined in a broader context, applying to all areas of human life (loss of health, loss of finances, loss of housing, loss of relationships, loss of an image of the future, etc.) Grief can be defined as an intense emotional state that accompanies the loss of someone/something with whom or which a person had a deep emotional connection.

In a person's current life, losses are part of reality, and not every loss will be traumatic. But living through a traumatic situation never happens without living through the loss and grief it causes. Losses can be divided into primary and secondary. Primary losses can be the actual traumatic macro-events that meet the criteria for diagnosing psychotrauma. And secondary losses can be the consequences of a traumatic event, micro-events that are not paid attention to because they do not have such an intense load as the main macro-event. But to heal psychotrauma, we also propose to process them using an integrative approach of positive psychotherapy [24, 25].

Symptoms

Traumatic experiences lead to a large number of well-understood physiological and psychological responses to threats that cause certain symptoms. Understanding the symptoms of trauma has a significant impact on the choice of trauma therapy strategy. Considering the specifics of working with a person in grief, N. Peseschkian identified four main forms of fear and depression in accordance with the four areas of life and four forms of conflict processing, which can lead to four *escape mechanisms* (into illness, into work, into loneliness, and into fantasy) (Fig. 31.2) [14].

We can also combine some specific human reactions to traumatic events into a table by areas of the balance model (Table 31.1).

The fear of death deserves special attention in the work with psychotrauma, which, according to N. Peseschkian, also has four dimensions:

- Fear of pain and suffering.
- Fear of losing one's profession and independence.
- Fear of separation from loved ones and at the same time fear of being a burden to family members.
- Fear of their own future.

N. Peseschkian advised to learn to distinguish between death as a predetermined fate and the attitude to death, which is formed through education, experience, cultural field, worldview, and religion, as a conditioned fate. C.G. Jung considered neurotic symptoms not only a defense against life traumas, but also an unconscious effort to heal them [5]. Therefore, knowing what symptoms and reactions a person may encounter, a psychotherapist can understand their needs, turn the patient to understanding their reactions, and draw up a therapeutic plan for the necessary interventions.

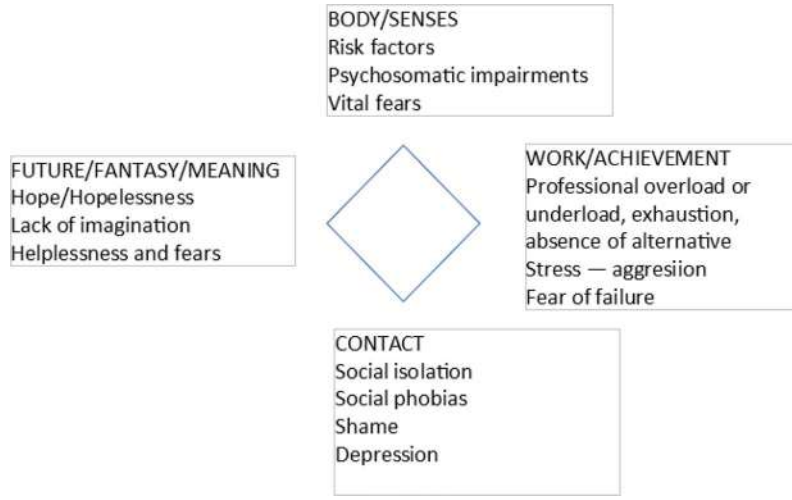


Fig. 31.2 The main forms of fear, depression and grief. [14]

Table 31.1 Symptoms of trauma according to the balance model

Body/senses	*Achievements/cognition and behavior*
Heart rhythm disturbances Heart not synchronized with breathing Body contact disturbance Feeling tiredness Decreased sexual activity Sleep problems Reactions "Fight", "Flight","Freeze", "Submit" Dissociation	There is no comlete coherent memory, only fragmented Loss of feeling of time Lack of words Lack of impulse control Agitation or immobilization Attention deficit disorder Catastrophization Protective negative concepts (illusions)
Contact/emotions	*Future/imagination*
Silent horror Rage Shame and guilt Perception through emotions Feelings of abandonment and loneliness Alienation Loss of connections Loss of trust	Loss of hope Destruction of the world picture Loss of the image of the future Paranoid fantasies Fear of death The spiritual aspect is violated The sense of purpose is destroyed Conflict of values

Principles of Positive Psychotherapy

If we summarize the main features of experiencing a traumatic situation from the point of view of positive psychotherapy, a person who has gone through a traumatic experience often feels hopeless, experiences an imbalance in their usual way of life and a sense of helplessness. Therefore, the principles of Peseschkian's positive psychotherapy: hope, balance, and counseling, are helpful for positive coping with stress and trauma in everyday life. We propose to consider them in more detail.

The Principle of Hope

Besel van der Kolk points out that the problem of psychological trauma is not just that a person is stuck in the past, but sees an equally big problem in the fact that they cannot live a full life in the present. Therefore, people who have experienced traumatic experiences often bring to the therapeutic session different complaints about conflicts in various areas of everyday life. Nossrat Peseschkian noted that a person's attitude to their disorders is determined by their attitude to the future: if the meaning of life and the image of the future are lost for them, then due to anxiety or apathy new crisis situations and PTSD reactions may arise [16]. In positive psychotherapy, 20% of attention is paid to the past, 30% to the present, and 50% to the future. From the point of view of positive psychotherapy in any area of behavior, the basis of stress is actual capacities. [17]. The cultural and social framework in which a person grows up determines the basic model of behavior

and reactions to stressful situations, which consists of a unique set of primary and secondary capacities for each person. In order to realize the principle of hope, N. Peseschkian considered it important to develop such capacities as hope, trust, and faith. N. Peseschkian defined hope as the capacity to develop a positive attitude toward one's own abilities, partner's abilities, and society beyond the present moment. It is aimed at the future. Through hope, a person learns to trust certain abilities that he or she has and that are expected of him or her. The so-called basic trust to the world is also important.

> Hope is not the same thing as optimism. It is not the conviction that something will turn out well, but the certainty that something makes sense, regardless of how it turns out.
> Vaclav Havel Source: (https://www.vhlf.org/havel-quotes/disturbing-the-peace/).

The development of a person's actual capacity of hope depends on his or her life experiences, emotional impressions, and the opportunities provided by the world around him or her. Hope as a connection to the future is controlled by positive experiences and disappointments that correlate with specific actual capacities. Hopelessness can manifest itself in different ways: from pessimism to despair [16]. Faith, according to N. Peseschkian, is the actual capacity to realize that you can rely on someone and feel protected there. It is the ability to rely on and expect certain achievements and qualities. The faith we have in ourselves as adults reflects the faith others have in us [16].

The use of the technique of positive reinterpretation of traumatic situations by viewing them as a challenge, as a lesson, and not as a punishment also correlates with Antonovsky's salutogenetic approach. If instead of "Why did I do this?", we ask ourselves the questions "What does this situation teach me?", "Why do I need this?", "When I learn this lesson, what opportunities will open up for me in the future?", this opens the way to get out of tunnel thinking and into an expanded vision of the situation from the perspective of the future. Tugba Sari describes the process of post-traumatic growth as a process of restoring balance in terms of PTSD [20].

The Principle of Balance

We propose that in working with trauma, the importance of a balance between the four areas of life, between the extremes of politeness and openness in the basic conflict model, between actual capacities in internal conflicts, and between concepts and counter concepts should be considered more broadly. The model of balance in positive psychotherapy is defined by four areas and qualities of life [15, 17]. In order to cope with the consequences of trauma and synthesize new energy, it is of great importance to restore balance between these four areas of human life. Having a supportive environment where you can safely share your story is essential for healing trauma. Effective interpersonal communication is based on a balance of relevant capacities.

The Key Conflict

According to N. Peseschkian, the dichotomy of politeness/appropriateness and openness/honesty, which the author calls the key conflict, is of particular importance. Politeness/appropriateness means recognizing generally accepted norms of interpersonal relations, neglecting one's own needs and interests in relation to the needs and interests of other people, and discouraging social aggression [16]. Openness, on the contrary, means defending one's own needs and interests without taking into account the interests and needs of other people. Traumatic situations often provoke a regression of mental functioning, which can manifest itself either in excessive politeness and emotions of fear, or excessive honesty, or in their ambivalence and sharp fluctuations. This is due to the fact that under the pressure of a charged internal conflict, the core of which is trauma, mature defense mechanisms are deformed, and immature ones begin to dominate. In regulating this phenomenon, an important role is played by understanding the origin of the imbalance of the key conflict of openness/honesty—politeness [7], which is part of the basic conflict of the personality and underlies any psy-

chosomatic symptoms. [16]. The key conflict of politeness—openness is also clearly manifested in the symptoms of protective reactions to a psychotraumatic situation. The "Fight" reaction can be seen as an instinctive manifestation of the actual capacity of honesty, the excessive manifestation of which can be perceived as aggression and contain the basic emotion of anger, and the "Flight", "Freeze", "Submit" reaction as a manifestation of the actual capacity of politeness, which is based on the basic emotion of fear. Earlier, we have already proposed to consider the way actual capacities are used in relation to others and in relation to oneself, so now we can talk about four ways of extreme expression of these actual capacities. These manifestations can be seen in the reactions of attack and avoidance, which Nathanson, D. L. sees as a manifestation of the feeling of shame [11]. Thus, by integrating the model of basic conflict with the model of four scenarios of shame (The Compass of Shame, [11]), we have developed a visual model of extreme degrees of politeness and openness with possible reactions to conflict in positive psychotherapy (Fig. 31.3).

Too much openness and too little politeness shown to other people will look like an attack, either physical or emotional, in order to improve one's sense of power and self-esteem (escape into contacts, escape into activities). Too much openness and too little politeness to oneself can manifest through physical or emotional autoaggressive manifestations of anger (escape into illness). Too much politeness and too little openness towards others can cause the desire to avoid expressing feelings and needs for communication with others and to isolate oneself from contacts (escape into activity, escape from contacts). Too much politeness and too little openness to oneself can manifest itself in the prohibition of feeling certain emotions and realizing the reason for one's own reactions. This can take the form of dissociative symptoms, which often arise as a defense mechanism for surviving a traumatic situation, but which interfere with conscious living in everyday life. It is the feeling of shame that is the main support for the defense reaction of dissociation from one's own inability to cope with extreme stress. In addition, the suffering of people who have lived through severe traumatic experiences such as the Holocaust, warfare, captivity, rape, etc. is often exacerbated by political events that do not allow for the creation of con-

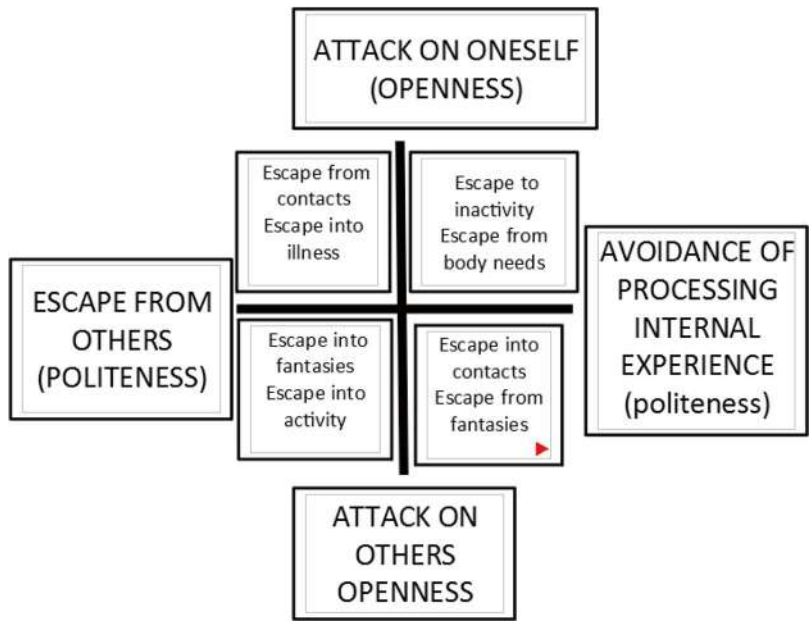


Fig. 31.3 Integration of the model of four scenarios of shame. [11] and the model of key conflict by Peseschkian [12]

ditions for collective social witnessing and response to trauma [3, 22] (the actual capacity is politeness). *"I can't tell my wife about my experience because I don't want to traumatize her. I can't tell my friends because they won't understand me because of their beliefs"—a patient who was held captive as a civilian in the temporarily occupied territory for about two weeks.* Excessive reactions to any situation due to impulsiveness or aggressive intentions (actual capacity—openness), as a way to cope with traumatic experiences, on the contrary, can lead to interpersonal conflicts as perceived by others as inadequate behavior and cause a desire to withdraw from society. *"Any little thing irritates me. I was ready to beat up the father of the girl who refused me a date"—a patient-soldier who came to the clinic after combat operations.* Thus, restoring the balance of politeness and openness will contribute to the healing of trauma.

Concepts and Basic Protective Illusions

The idea of concepts is one of the most central positions in positive psychotherapy. In positive psychotherapy, concepts are defined as cognitive and emotional structures that provide a framework for interpreting relationships and attitudes toward oneself, others, the environment, and the world. The concepts are based on a triad that includes *cognition, emotions, and behavior*. The content of the concepts is described in terms of basic and actual capacities, and their place in the system of connections and relationships is described in terms of four areas of conflict processing, model dimensions, and forms of interaction [12]. The main goal of psychodynamic psychotherapy is to help a person understand the internal dynamics of the hidden causes of traumatic reactions and to find ways to resolve the unconscious conflict that is exacerbated by traumatic experience. Y. Medynska notes that in the work with PTSD, the conflict between the discrepancy between a person's own values, his or her picture of the world and the surrounding reality of the traumatic situation is often considered [9]. Therefore, when working with traumatic experience, it is important to study the system of concepts and needs of a person and how the behavior and experience of the traumatic situation affected them.

In positive psychotherapy, a person's behavior and reactions are determined by basic and actual concepts. Basic concepts are established in childhood through parents' attitudes toward the child, each other, other people and groups, the world, and the philosophy of life. These concepts relate to the four areas of a person's life that are described in the balance model in positive psychotherapy: body, achievements, contacts, and future/fantasy/meaning. Actual concepts relate to current situations in which they determine a person's attitude and response. These life situations can occur in different areas of the balance model [24]. Concepts in positive psychotherapy are behavioral navigators. They are related to feelings and, in case of a conflict between expectations and reality, can be a source of fear and depression [16]. How and to what extent a person experiences a hopeless situation depends on the upbringing he or she received, which in positive psychotherapy is called basic conflict [16]. From birth, in the process of development, an individual tries to satisfy his or her basic needs and structures his or her perception of the world around him or her and the image of his or her own self through basic concepts. It turned out that from childhood, a person develops some basic ideas about the existing world and their future in it, which are mainly protective in nature and have no confirmation by reality [19]. We assume that this process takes place in the future/fantasy/meaning area and with the help of the actual capacities of faith and trust. These representations can be stored in the individual conceptual program as basic protective concepts in relation to different areas of life until adulthood, not realized as basic illusions and have content in the form of actual capacities. Combining the theory of basic protective illusions [19], the theory of basic needs of the child and our experience of working with psychotrauma, we identified four

types of protective basic illusions in terms of four areas of conflict processing.

The Body Area—The Illusion of One's Own Immortality and Invulnerability

The basis of this illusion is the need for safety, the need to avoid pain, and the desire for pleasant sensations (the actual capacity of trust in one's body and basic trust in the world, hope, and faith). It is difficult for a person to realize that they will die. A person is afraid to feel pain, so he or she chooses to believe that nothing of the things that are reported in the news will happen to him or her.

Area of Achievement—The Illusion of Control

The basis of this illusion is the need for control and orientation (actual capacity—achievement, performance expected of oneself and others, obedience/discipline). People need to influence and navigate the world around them. A sense of control comes from having a choice of several options. It gives a sense of involvement in life and a sense of security.

Area of Contacts—The Illusion of Infallibility of One's Own and Other People's Behavior and Expectations of Favor from Other People

This illusion is based on the basic need for attachment (actual capacity—love, trust, unity). A person wants to feel good about themselves and feel the affection of other people, to be sure that they are needed, loved, and want to feel that they are not alone. It can also be an idealized positive image of other people and expectations of their attitudes and behavior towards themselves and others.

Area of Fantasy and Meaning—The Illusion of the World Around Us as Orderly, Cmprehensible, and Just

In order to feel inner peace, a person needs to believe that the world around them has its own rules, its own order, its own justice, and that everything that happens can be explained and comprehended (the actual capacity of hope, faith, justice, orderliness). The extreme stress that results from a traumatic event destroys these basic concepts, radically changing the picture of the world. Then, protective illusions can change their pole and involve fear, anxiety, and avoidance strategies for additional illusory protection [25]. Also, negative protective illusions can be formed as basic concepts due to negative childhood experiences or negative transcultural experiences of past generations [8].

We propose to consider examples of positive and negative protective illusionary concepts and actual capacities that are the content of these concepts according to the type of illusion (Table 31.2):

It is possible to see the content of the most common internal conflicts in individuals who have experienced traumatic experiences and attempts to resolve them through the dichotomy of activating actual capacities. If the model of the world and the self-image is based on basic protective illusory concepts, then a traumatic event

Table 31.2 Positive and negative protective illusionary concepts and actual capacities

Types of illusions	Positive protective concept	Negative protective concept	Actual capacities
The illusion of one's own immortality and invulnerability	"I will never die" "This can't happen to me"	"I will die" "This is the end" "I could die" "Something can happen to me"	Trust Faith Hope
The illusion of control	"I control everything" "I have to have everything under control" "I have an influence on it"	"I have no control over anything" "Nothing depends on me" "I can't influence anything" "I am powerless to change anything"	Obedience/ discipline Achievement/ Diligence Reliability

(continued)

Table 31.2 (continued)

Types of illusions	Positive protective concept	Negative protective concept	Actual capacities
The illusion of infallibility of self and others and expectations of favor from others	"I am good" "I am a good person" "I am the best" "Everyone loves me" "I love everyone" "Every person is good"	"I am a bad person" "I am a monster" "I am an evil person" "I am a monster" "Nobody loves me" "I am not worthy of love" "People are evil" "I hate everyone" "The more I get to know people, the more I love dogs"	Love Contact Confidence Unity
The illusion that the world around us is orderdered, comprehensible and just	"The world is a good place" "The world is a safe place" "The world is a just place" "The world is my friend" "The world loves me" "If I behave properly, the world will protect me"	"The world is an evil place" "The world is a dangerous place" "The world is a threatening place" "The world is not fair" "The world is my enemy" "The world is calling me to battle" "The world has betrayed me" "Why did the world punish me?" "Why did this happen to me?"	Justice Orderliness

Table 31.3 Concepts with patients who have experienced traumatic events

	Internal unconscious conflict	Negative protective illusory concept	Extended actual concept
A military man, combatant, who was treated in a hospital	Control (efficiency) - trust	*"I can't feel safe without a gun "*	*"I am strong enough to take care of myself"*
A mother who lost her son to suicide	Justice - Love	*"The world is unfair. The world closed in on me. My life was over"*	*"The world is neutral. It turns out that it was me who closed myself off from the world. And life goes on"*
A woman whose husband went missing in the war	Justice—Love	*"I thought this would never happen to me. I will not survive this"*	*"It happened to me and I have to get over it"*
A woman who had to leave her home because of the outbreak of war	Control—trust	*"My relatives are me enemies, they want to harm me"»*	*"My loved ones love me and are worried too, they just don't know what I feel. They are my support and my strength"*

makes it impossible to reflect the existing reality as it is. The person finds himself or herself locked in the grip of his or her false beliefs. It is necessary to devote time and attention to establishing a sense of inner security, rethinking, resolving internal unconscious conflicts, balancing actual capacities, and expanding concepts about one's own picture of the world, the environment, other people, and oneself. N. Peseschkian called an alternative concept that can help in resolving a difficult situation "psychological serum" [13]. Here are the results of the work on expanding the concepts with patients who have experienced traumatic events and suffered their devastating effects (Table 31.3):

The Principle of Counseling in PPT

In the process of living with psychological trauma and adapting to a new life, the five-stage model of therapy by N. Peseschkian is used, which can be

used as a process within each session and as a general strategy throughout therapy [17].

1. The *first step* in the process is *acceptance, observation, and distancing***,** which involves changing the perspective on the situation, on the traumatic experience, and on the experience of loss.

   The therapist's tasks are to listen and observe the patient while he or she talks about the traumatic experience and try to separate it from the painful experience in order to look at the situation holistically.

It is also important to pay attention to the following points:

- Assist, but not force, the client to talk about what happened.
- Provide an opportunity to speak without being interrupted.
- Encourage storytelling, guide the conversation.
- Helping a client express feelings.
- Show patience. Give time to respond.
- Investigate how long the problem has existed.
- Clarify the patient's perception of the traumatic situation.
- Ask what the patient thinks about his or her actions in a traumatic situation.
- Assess the degree of suicide risk.
- Helping to look at the situation objectively.
- Helping the client to see themselves as a whole apart from the traumatic situation.

Therapeutic relationship: establishing contact through the area of future/fantasy/meaning, hope, creating a safe space, positive vision of a person implies faith in his/her abilities and sufficient resources to cope. The ability to withstand and not be shocked by trauma. To see the personality behind the wound. Be able to cope with traumatic countertransference.

At the same time, it is necessary to take a holistic view of the situation and make a realistic assessment of the presence of traumatic factors:

- Was it unpredictable?
- Was it uncontrollable by the client?
- What/who was the threat and how strong was it?
- Was there a threat to human life/health?

It is important that the patient receives a real objective assessment/opinion about the situation with validation of his or her feelings and reactions to dispel feelings of shame and guilt. *The following techniques can be used:* active listening, grounding, metaphor, parable, transcultural approach, third-person narration, positive reinterpretation, drawing, placement, etc.

Examples from practice
The client, just over 40 years old, experienced the traumatic loss of her only son to suicide. She blamed herself with obsessive thoughts about her own suicide as an atonement for her guilt.

T: "I'm going to tell you a parable about the 'Glass Sarcophagus' [17] What do you think of it?".
Cl: "I think this 'sarcophagus' is in my apartment. I don't want to let my son go. But still, one day I have to do it" (said with great regret).
T: "What did you realize, what conclusions did you draw after the experience?".
Quote: "The most evil organ of the human body is the tongue. Life and death are at the tip of the tongue" (key conflict politeness—openness).

2. The **second** stage is an inventory that differentiates the content of different types of conflicts and the patient's strengths.

The therapist's tasks are to study a person's reactions to traumatic experiences and to study the mechanisms of conflict (conflict and resource map). It is also important to pay attention to the following points:

- Investigate the conditions for the problem formation.
- Explore the current state of the problem and related feelings and reactions.

Possible areas and techniques of work:

- What macro and micro events have happened to the client in their entire life or during a cer-

tain period of life? (trauma line and resource line).
- Identify the event or experience that triggered the acute crisis or PTSD.
- Identify which situations in the present trigger experiences from the past.

> "I started dreaming about the war. I dream about the battlefield with my dead comrades. Did I tell you that I killed a man—a sentry from the enemy?"—patient, demobilized soldier who witnessed a fatal accident

- The balance model: research during the event, immediately after the event, and now:
- How did this experience affect a person's opinion of themselves, others, and the world?
- How did this experience change the sensations in the client's body?
- How did this experience affect the client's thinking and behavior? (Hyper-arousal, avoidance, intrusions, etc.)
- How did this experience make you feel?
- Reflection of the quality of life:
- How did this experience affect a person's values and vision of the future (negatively and positively)?
- How did this experience affect your body and health (negatively and positively)?
- How did this experience influence your area of work and achievement (negatively and positively)?
- How did this experience affect your relationships with others and yourself (negatively and positively)?

According to DAI:

- What actual capacities were manifested in this situation in active and passive forms (conflict potentials and resource actual capacities)?
- What actual capacities were missing?
- What are the identified actual and basic concepts (protective illusions and resource actual concepts)?
- What was the situation in childhood (basic conflict and basic resource)?
- At what stages of interaction with trauma, grief, a lost person, or an object is the patient?

It is also important to pay attention to the following points:

- Map the conflict and the resource.
- Reformulate the problem for the patient based on the conflict and resource map.
- If the problem is large, divide it into parts and prioritize the focus of attention.

3. In the *third* step, called situational encouragement, the patient's resources and skills are actualized and developed.

The therapist's task is to stabilize the patient's condition while activating resources. It is also important to pay attention to the following points:

- Find out if the patient has experienced something similar before and what helped him.
- Find out if there is anyone in the environment who can provide support to the patient.
- How can a patient provide support to themselves.

Possible areas and techniques of work:

- Resource psychoeducation.
- Therapeutic relationship as a factor that gives a sense of understanding and security (curiosity, empathy, congruence, and acceptance).
- Stabilization techniques.
- Metaphor, transcultural approach, positive reinterpretation.
- Training in breathing techniques.
- Resource search (Past-Present-Future).
- Search for resource relationships.
- Visualization of "Safe place", "Safe state", etc.
- Find personality strengths in different identities.

4. Then the current situation is processed at **the fourth** stage—verbalization.

The therapist's task is to look for ways to recover.

It is also important to pay attention to the following points:

- Invite the patient to a verbal discussion of plans and possible options for behavior.
- Consistently discuss possible options for a person's actions in the coming days.
- Create an "anti-crisis plan" in case of failure.
- Look for alternatives for every important topic.
- Discuss together the pros and cons of the planned actions.
- Promote the rejection of unrealistic/antisocial ideas.
- Stop the desire to control events that the client has no control over (actions and feelings of other people, situations beyond the client's control, etc.)
- Support client choice as an ability to influence what is happening and transfer responsibility for their own actions

Possible areas and techniques of work:

- Motivation for recovery: the positive or negative consequences for significant others of choosing work or not to work on recovery.
- Dealing with losses (primary and secondary).
- Recognize and manage triggers.
- Grounding techniques.
- Body practices.
- Lessons learned from the experience.
- Adopting experience into your life.
- Techniques for forgiveness.
- Planning resource activities based on the balance model: at home, on the street, in society.
- Restoring the balance of politeness and openness.
- Balancing actual capacities.
- Use of transcultural experience (concepts, actual capacities, actions, etc.) from one's past to solve problems of the present.
- Expanding concepts.
- Working with metaphorical parts of the personality (theory of structural dissociation by Onno van der Hart).

5. The *fifth* step, called goal expansion, involves future-oriented reflection, synthesis, and testing of new concepts, strategies, and perspectives.

The therapist's task is to discuss the meaning of the future and plans for it.

Possible areas and techniques of work:

- An image of the desired future.
- Transferring responsibility for contributing to the realization of the desired today.
- New energy distribution.
- Message to yourself from the future (letter, chair from the future, visualization).

The 5-step model of therapy and self-help in positive psychotherapy provides both therapist and patient with a starting point, as well as a sense of security and prepares the patient for self-help in dealing with conflicts, especially after therapy.

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32 Sexual Counseling and Sexual Education Using Methods of Positive Psychotherapy

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Background

This chapter addresses the development of sexuality according to the model of positive psychotherapy [8]. Sexuality-related problems frequently come up as themes in therapy and this chapter is meant to be a tool for prevention and intervention.

Positive Psychotherapy also analyzes our development as individuals through the perspective of the basic capacities and resources we learn during childhood. Thus, we are equipped with two sets of capacities—primary capacities (love, attachment) that we learn at the age 0–3 through the relationship with the attachment figures and secondary capacities (or knowledge capacities) that are rather norms and rules which develop in social interactions. Depending on our relationships and interactions, we develop our capacities to a certain degree, on a continuum between two extremes: one being the poorly developed form of the capacity and the other being the exaggerated development of the capacity. The average values represent flexibility, a harmonized capacity, while the extremes are those that usually bring us into conflict with others.

Let's take as an example the primary capacity of *time*. If during his attachment period, when this capacity is structured, a child does not spend much time with his attachment figures (he is often left alone, and when he asks for adults he is quickly dismissed), he will most likely develop this mental model, the following pattern about relationships in general: namely that it's natural to have very few friends, that you generally must deal with your problems by yourself, that you don't need to socialize for a long time and that you can spend most of your time alone, without other people around. And if, as an adult, such a person ends up in a relationship with someone for whom time is a very important dimension (who has experienced the exact opposite model, in which the adults always had time for their needs and there was a lot of interaction, etc.), turbulence and conflicts will naturally occur between these two. And this is not because the former is careless while the latter is extremely involved in the relationship, but simply because they've had different experiences and developed different mental models about relationships and the world in general. The same is true for any other capacity listed in the figure below (Table 32.1).

Professor Nossrat Peseschkian, the founder of Positive Psychotherapy, developed an assessment tool—WIPPF (Wiesbaden Inventory of Positive Psychotherapy and Family Therapy, [9]) which investigates the degree of development of these capacities and the way in which certain primary and secondary capacities have conditioned one another during our childhood experiences; for

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_32

Table 32.1 The basic capacities of humans according to Positive Psychotherapy

Basic capacities—positive psychotherapy [8]		
Underdeveloped	Capacity	Overdeveloped
Primary capacities		
Inpatient	Patience	Hyper-patient
Over-requesting	Time	Under-requesting
Lack of contact	Contact	Contact maniac
Suspicious	Trust	Blind trust
Hopeless	Hope	Naïve, disadaptive optimism
Refusal	Tenderness/sexuality	Addicted
Constantly asks for	Love/acceptance	Constantly gives
Impassible	Faith/meaning	Categorical
Secondary capacities		
Untidy	Orderliness	Shipshape
Sloppy	Cleanliness	Sterile/hygienic
Unpunctual	Punctuality	Over-punctual
Lack of consideration	Politeness	Over-adaptable
Dishonest	Openness/honesty	Excessively honest
Inactive	Diligence/activity	Hyperactive
Lack of credibility	Conscientiousness	Perfectionist
Wasteful	Thrift	Cheap/avaricious
Non-authoritarian/submissive	Obedience/submission	Authoritarian
Unjust	Justice	Justice-driven
Unfaithful	Fidelity	Dependent

example, *love, acceptance* (primary capacity) and *diligence, activity* (secondary capacity). If a child is repeatedly induced the idea that he doesn't deserve to be loved unless he is diligent and works hard (not necessarily explicitly stated, but the mere attitude of the attachment figures towards this association can produce effects), these two capacities become conditioned based on the principle "if... then" and forms what Positive Psychotherapy calls "basic conflict". Conditioning "love, acceptance" to "diligence, activity" is just one example, all kinds of conditioning can occur between our primary and secondary capacities.

The characteristic of the basic conflict is that it settles in the subconscious and, without us realizing, it ends up influencing the choices we make, the way we relate to ourselves, to others, to work, to life in general. Most often, such people (with a conditioning between love, acceptance and diligence, activity) are highly career-driven, they make extraordinary efforts in the performance area and become exhausted, simply because an internalized "voice" constantly tells them "you deserve the love and acceptance of others if you perform, if you work very hard"; and their brain will automatically make the opposite inference: "If you don't work very hard, you don't deserve to be loved and accepted by others." This does not mean that Positive Psychotherapy sees performance as useless, non-essential, but only that love and acceptance should not be conditioned by it. In fact, love and acceptance should not be conditioned by anything.

These clarifications about the views of Positive Psychotherapy will assist to understand the complex path, the process by which sexuality develops throughout life. Our sexuality is not non-existent before puberty and does not appear "out of nowhere" at this age. Perhaps those who disagree with this idea and consider that sexuality begins at puberty, believe that it's unnecessary to approach sexuality before puberty and that anatomy provides all the answers. But anatomy should be addressed before puberty, to prepare children for the transformations they will strongly feel in their bodies. We all know cases when, at the first menstrual cycle, some girls were

extremely scared, they were convinced that they were seriously ill and would die. In some situations, lack of information produces panic or worse: it pushes children to search for information on their own, on various dangerous sources.

Returning to the capacities that are closely related to the development of sexuality, we notice in Table 32.1 that the *primary* capacities (which are formed at the age 0–3) are almost all related to our sexuality. The way we experience love, acceptance, contact, tenderness, trust in relationships with attachment figures, will also mark the way we will later relate to sexuality, the type of sexual behavior we will engage in or the way we will treat our partners in sexual and couple relationships. The same is true for the secondary *capacities*, which develop after the end of the attachment period (age 3–7) through social learning and regulation. At least two of them (obedience/submission and fidelity) clearly call for our attention through their direct involvement in the development of our sexuality. The way we directly experienced these two capacities as well as the way we saw them manifest in the relationships around us (with parents, close relatives)—the "model dimension", become patterns that we will apply in our own relationships. Of course, there is also the possibility of developing the countermodel, but most often we repeat exactly the patterns known from childhood, even when they are dysfunctional and harmful. Perhaps some of us are familiar with the expression: "I will never do what my parents did!" and then end up repeating the model precisely.

When analyzing the sexuality dimension of a person, it's better to go deeper than just sexual behavior: How did the person experience love, tenderness, contact, faith, spirituality, how did he learn fidelity, or relationships through power, submission or domination, etc.

The approach to sexuality from the perspective of Positive Psychotherapy involves exploring all four areas of the Balance model: body-senses, performance/knowledge, social relationships, and fantasy/future/spirituality.

Addressing sexuality along the developmental path, it should be noted that by *preschool*, most children have already developed a strong sense of their own gender (of being a boy or a girl). They will continue to explore their own bodies, often intentionally [5]. In such situations, physical, penalizing corrections or aggressive language does not help, they will not stop these explorations but will induce feelings of guilt and shame. Instead, these situations can represent excellent opportunities to learn, for example, *the difference between public and private or appropriate touches-inappropriate touches*.

First, it is essential that we investigate the child's level of understanding and adapt our language and explanations to it. The proper approach to sexuality is never a one-off discussion, but it is a process, a series of conversations that are periodically resumed and that are triggered especially when the little ones ask questions. Keep in mind that if, when they have a question, we don't answer it (we dismiss the question, we deviate from the subject, we tell them that one must not talk about such things), their search will not stop and the desire to find answers will not be extinguished, but they will turn to other sources that can be dangerous. Moreover, the conversation should be stimulating, and give them the opportunity to ask more questions so that we can make sure they understand. In this particular situation described above, our goal is to make children understand that some things are not meant to be experienced in public and that no one, not even family members or other close people should ever touch them in ways that make them feel uncomfortable, and if this somehow happens, they should know that they can notify their parents or another trusted adult to ask for help.

Also, during this period, you may be asked where babies come from or why girls don't have a penis. The recommendation is to answer honestly and calmly. The story with the stork often creates confusion when it is used to substitute or twist a biological truth. Sexuality also has a spiritual side that we cannot ignore. The stork is a symbol of the angel announcing the arrival of a child, an idea that does not exclude, but rather completes the biological reality of procreation. If you tell children that the stork brings babies and with that you end the discussion, you can put them in the situation of being mocked by their

peers if they present such an explanation and, in the end, your credibility in front of them will have to suffer. Answering them honestly right now is encouraging them to come back to you with other questions in the future. And in this way, you keep an authentic image of what your child knows and experiences in terms of sexuality. On the other hand, when answering such questions, we do not have to provide many details, which are not necessary at this age. It is important to find out exactly what they want to know and to answer specifically. For example, we can tell them that a man and a woman can have a baby and that the baby grows in the woman's belly. There is no need, at this point, for any other additional information about how children are conceived when talking to preschoolers.

Next comes the stage when the children become curious about the bodies of others [5]. Playing "doctor" is becoming more frequent, and this is another situation in which it is very important not to react impulsively or aggressively if you find them naked. Of course, if such a game involves a preschooler and an older child, this situation is not appropriate and must be approached specifically, but still calmly, in order to find out what actually happened and make sure that it will not happen again (a resumption of public-private themes, appropriate touches—inappropriate, with both playing partners). In situations where young children of the same age are exploring their bodies, you can subtly distract them by involving them in other activities. But it is very important to see this moment as an indication that the curiosity to explore others' bodies has been activated and that this curiosity and learning can be facilitated and shaped in other ways, through age-appropriate books about the human body or through drawings of the body that you make together with them.

The preschool stage is an age of imitation [7], and we must remember that children naturally imitate what adults do, not what adults tell them to do. Thus, they will imitate exactly our sexual attitudes: the way we relate in relationships, how we react to the opposite sex, our beliefs about others, about nudity, etc. Also, on the principle of imitation, you may hear them talk about the fact that they fell in love, that they have boyfriends or girlfriends, that they want to get married, etc. Keep in mind that at this age they do not give words the meaning and significance that we give to these concepts, so there is no need to panic. The most appropriate attitude is a neutral one, through which we neither encourage these behaviors, nor overreact.

At *the beginning of the school period*, children become interested in pregnancy, childbirth, and gender roles. At this stage, boys prefer to play with other boys, and girls with other girls. At the same time, other colleagues and the media (especially online media) are starting to exert a great influence on their sexual attitudes. If the child does not see the parent as a source of trust and information, he will turn to colleagues (often older) or to the Internet for information related to sex, sexual organs, reproduction, etc. and the chances of accessing inaccurate or inappropriate information are very high [3].

If by this age the child does not ask anything about sexuality, it is important to initiate such conversations, appropriate to their age. The simple explanation given at the preschool age about how a woman and a man have a baby, can now be nuanced, developed, and reinforced with more information. It is important to provide correct and adequate information, because if not, children often tend to draw their own conclusions, sometimes inaccurate. They may assume, for example, that babies are made when a man and a woman sit next to each other, if they sleep in the same bed, if they hold hands, if they kiss or swim together, etc.

During this age, certain maladaptive behaviors or attitudes may appear. *Offensive language* may be used, for example, taken from TV programs or movies, from friends, or in some cases even from the family. Children often use words or make inappropriate jokes without understanding their meaning, on topics related to sex, body parts, sexual orientation, etc., and without realizing they may be hurtful. It is best to react calmly, to explain why those words or jokes are inappropriate and, above all, to offer healthy, suitable alternatives for the future. In such situations, us adults might want to analyze our own prejudices

expressed at home, out loud, through ironies against certain social categories or people. Without realizing it, we could be the source of such inappropriate behaviors. Just like bullying, tolerance and respect are also learned behaviors and attitudes.

As children grow up and begin to show signs of *puberty***,** our attitude and acceptance as adults play a very important role in their healthy development. At this age they go through actual emotional storms and experience contradictory feelings at the same time. They may be equally excited and frightened by the transformations in their bodies (voice change, breasts growth, body hair growth, the first period, spontaneous ejaculations, nocturnal emission, etc.), and they spend a lot of time comparing themselves with their friends and wondering if these changes are normal.

Adolescence (with its stages—early, middle, and late) often brings conflicts between parents and children, opposing attitudes, the need for autonomy and independence, and anxiety about defining one's own identity. All this "turbulence" should not prevent from approaching topics related to sexuality in the family. At this age, correct information about pregnancy, means of protection, or sexually transmitted infections is vital. In other words, the goal is to make adolescents understand that sexual behavior has consequences and that their safety and health should come before pleasure and experimentation [3].

We also approach the idea of *continuum*, the successive stages of intimacy—from time spent together to holding hands, hugs, kissing, and sexual intercourse. This is also a very good time to bring up our moral and spiritual values, as a family, in relation to sexuality. It is very important to give them our own opinion and perspective, but at the same time to avoid activating and expressing our own prejudices about sexuality.

In the process of sex education and counseling, relevant topics from five main areas are addressed: notions related to the human body, relationships, sexual behaviors, parenting, and the issue of violence. The themes included in each area refer to the entire educational path, so not all of them will be addressed in each level of education, but, depending on the age of the children, some topics will be addressed, others will be kept for later. For example, from Chap. 1. *My body,* in kindergarten and in the first years of school we will address the first topic—*Discovering the human body* but not *Puberty—menstruation*.

1. A schematic presentation of the fields and themes addressed, the effectiveness of which is shown in the literature [1, 2, 6] is given below:
2. ***My Body.***
   - Discovering the human body.
   - Transformations in our body.
   - Puberty—menstruation.
   - My Health.
   - Self-esteem—positive self-image, self-confidence.
   - Beauty and attraction.
   - Enjoying your body.
3. ***Relationships.***
   - Socializing.
   - Attraction.
   - I'm seeing someone, I've got a partner.
   - Relationship and sex within the relationship.
   - Family Model/Family/Marriage.
   - Situations of separation, breakup.
4. ***Sexual behaviors.***
   - Myths about sexuality.
   - My sexuality.
   - Masturbation.
   - Means of protection.
   - Sex with other people.
   - Sexual orientation.
5. ***Parenting.***
   - Pregnancy.
   - Birth.
   - Having a baby and taking care of a child.
   - Adopting a child.
6. ***Violence.***
   - Bullying.
   - Sexual harassment.
   - Sexual abuse.
   - Domestic violence.
   - Institutional violence.
   - Child sexual abuse.

Parents or guardians, sometimes even teachers or counselors, are often fearful or reluctant to approach these topics and it is perfectly natural to feel uncomfortable. But this reluctancy must be overcome if we expect our children/clients to exhibit appropriate and responsible sexual behaviors.

These fears can come up in various ways. Parents becoming anxious at the first signs of sexual expression of their very young children, anxiety about physical transformations, genital hygiene, inappropriate sexual impulses, embarrassing situations in public, to the fear of an unwanted pregnancy, contracting STDs, rape, sexual abuse or exploitation. In addition, there are also worries about the way they should answer their questions about sexuality, what their children should know or already know on this subject. There is a very common conception that *"if someone has information about sexuality, they will engage in sexual activities, and if they don't have information, they will not engage"*. However, we already know that things are exactly the opposite, and if we do not provide correct and adequate information about sexuality, they can engage in inappropriate, dangerous sexual behaviors.

In the process of sex education and counseling, the first step is to assess the level of understanding of the person we work with. Depending on this, we will select the approach strategies and adapt the information we want to deliver.

Understanding that we cannot have a single discussion on this topic, we will establish certain moments in which we can address these subjects, and we build the discussion in such a way as to provide clients with opportunities to ask questions, to ask for additional information, to receive clarifications. It helps a lot to break down the topics into smaller, accessible topics and check the understanding after each such subtopic. Open questions such as "Did I answer your question?", "What do you think about what we discussed?", "What else would you like to know about this?" etc., help us see how the information was assimilated, but at the same time we build a relationship of trust between us and the beneficiary. And if we find that the subject has not been understood, we don't give up, but we will approach it differently, from another angle, at another time, broken down into even smaller bits. In addition to these generic suggestions in the approach, I present below some recommendations that I consider useful in this process:

For Parents

- It's important to analyze your own feelings about sexuality before talking to your child about it. If such topics seem immoral to you, you can talk to a counselor, or a priest and it is very possible that you will come to the conclusion that prejudices about sexuality have no foundation in either morality or religion.
- It is important to be aware that your son or daughter will find out about it anyway from colleagues, TV, or online. While information from such sources can be incorrect or dangerous, it is important to trust that you can be a much better educator.
- Approach these topics exactly as you would have liked someone you trust to have addressed them with you when you were a child or teenager. With this idea in mind, it is much easier to talk about the private parts of the body, about hygiene, menstruation, masturbation, puberty, sexual behaviors, contraception, marriage, pregnancy, etc.
- When in doubt about whether or not it is appropriate to address these issues, remember that if they do not have enough information, there is a risk of abuse. You don't want this in any way, so you need to talk to them about appropriate and inappropriate touching, about relationships, rape, harassment, abuse, and exploitation.
- Trust your child's ability to learn. It's all about finding the right strategies for its characteristics and level of development.
- Repeatedly insist on the idea that sexuality is an absolutely natural part of our being and life, but that sexuality is not just about having sex.
- Discuss calmly, patiently, and acceptingly, without stigmatizing or discriminating. Encourage children to ask questions.

For Teachers and Counselors

- Before starting to address these topics, self-assess the dissonance between personal beliefs and facts.
- We never approach sexuality only through the perspective of sex, we will always bring up the affective, relational, moral, spiritual elements related to it; that is, we will approach it in all areas of the Balance model.
- Insist and constantly resume the idea of responsibility for our body, for the actions and for the consequences of our actions.
- Show respect for their personal space, for privacy, and for the decisions they make in the education or counseling process, ask for their consent whenever necessary.
- Always check the resources you recommend to beneficiaries, especially those online. You may need to make your own, adequate, and personalized resources.
- Always use the official and correct words for body parts and sexual activities. You probably need to clarify certain misunderstandings or misinformation about these aspects.
- It is desirable that all school staff and counselors develop basic skills in managing situations related to sexuality and know exactly where they can guide beneficiaries for specialized counseling.
- We do not have to wait for crisis situations to arise in order to address them in counseling. A lot of information about appropriate social norms and behaviors can be provided even if embarrassing situations have not yet occurred.
- We periodically resume discussions related to appropriate versus abusive touches, public versus private, healthy versus unhealthy behaviors.

It is good for parents to understand that, whether they want to or not, they are the first sex educators for their children. In everyday family life, children learn about sex, sexuality, and relationships, through the verbal or nonverbal language of their parents. Sometimes families may not be aware of the impact of these influences, but in most cases, the decisions (also) about sexuality of young people and adults reflect what they have heard, seen, and experienced in their families. Thus, the alliance between parents and professionals is a crucial resource for a healthy development and expression of sexuality [4, 10].

As a conclusion, in communicating about sexuality it is recommended that you always start from the premise that those you address have the right:

- To ask any questions about sexuality-related topics that interest them: body, puberty, sex, reproduction, relationships, etc.
- To receive appropriate, medically correct information, adapted to their age and level of understanding of sexuality.
- To receive support in developing the skills necessary to form healthy friendship, romantic or couple relationships.
- To receive support from adults who accept and respect them unconditionally.

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Part V

Theoretical Foundations and Training

Theoretical Foundations and Roots of Positive Psychotherapy

33

Arno Remmers

Humanistic Roots

> It is as if Freud supplied us the sick half of psychology and we must now fill it out with the healthy half.
> Abraham Maslow [1]

In the first chapter about a "positive view" in treatment, Peseschkian explained in his book "Positive Psychotherapy" (1987): "While many of the existing psychotherapeutic procedures take the disturbances and illnesses as their starting point, prophylactic and preventive medicine and psychotherapy require a different method of proceeding, starting from the person's developmental possibilities and capacities instead of the disturbances." Abraham Maslow, the proponent of the hierarchy of needs and one of the founders of humanistic psychology, saw "…the importance of focusing on the positive qualities in people, as opposed to treating them as a bag of symptoms" [2]. Maslow was also the first psychologist to use the term "positive psychology" (1954). Humanistic psychologists "believe that every person has a strong desire to realize their full potential, to reach a level of self-actualization." The main point of humanistic psychology was "to emphasize the positive potential of human beings" [3] and as such "…the final level of psychological development that can be achieved when all basic and mental needs are essentially fulfilled and the 'actualization' of the full personal potential takes place [4]."

The idea for positive psychotherapy (PPT) arose in the 1960s, according to an oral message by Manije Peseschkian, the wife of Nossrat Peseschkian, on a flight to the USA in 1968. It was the period of the development of humanistic psychology and psychotherapy in the USA, pioneered by Abraham Maslow and Carl Rogers. The American Association for Humanistic Psychology (AHP) had been founded in 1962. Humanistic psychologists had been influenced by Kurt Goldstein, who saw *self-actualization* as "the tendency to actualize, as much as possible, [the organism's] individual capacities" … "At any moment the organism has the fundamental tendency to *actualize all its capacities*, its whole potential, *as it is present in exactly that moment, and in exactly that situation in contact with the world under the given circumstances*" [5]. "The curative force in psychotherapy–man's *tendency to actualize himself*, to become his potentialities… to express and activate all the *capacities* of the organism [6]."

The term "*actual capacities*," *used in PPT*, is in line with this humanistic tradition: "The roots of differentiation analysis reach back farther, to the schools of classical psychotherapy" [7]. Peseschkian describes "actual capacities, because

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_33

in daily life they are, in the most diverse ways, being continually addressed at every moment." In September 1974, Peseschkian had a lecture about the "Actual Capacities as Aspects of Connotation and Social Organization of Conflict Handling" [8], and in the same month he delivered a speech about "The meaning of norm conflicts in the development of psychosomatic diseases" at the 12th International Congress on Internal Medicine in Tel Aviv. The actual capacities have been worked out by factor analysis of psychotherapeutic interviews with patients from different cultures ([9], 1988). Peseschkian compared these terms with those used by other authors (1977), including Maslow's description of "self-actualizing people, that share the following qualities: Truth: …pure, *clean* … *honesty*, … *perfection*, … *unity*, integration, tendency to oneness, …*order*, … *acceptance* … *uniqueness*… *justice*…" and Engler [10] comments that "the usefulness of the concepts of self and self-actualization continue to attract discussion and debate." Peseschkian wanted to create a practicable, understandable, and systematic psychotherapy method using the terms of actual capacities for self-development.

Peseschkian's personal transcultural experiences drew his attention to the significance of psychosocial norms for socialization and the birth of interpersonal and internal psychological conflicts. He "found, … that in connection with the presenting symptoms there were conflicts stemming back to a number of recurring modes of behavior." In turn, "an inventory [was] drawn up, with the help of which the content components of the central conflict areas could be described. That which appeared as conflict potential and developmental dimensions in the spheres of child-rearing and psychotherapy was, in the domain of morality and religion, reflected in the normative sense as *virtues*. Out of the psychotherapeutically relevant behavioral and attitudinal norms was developed, the differentiation-analytic inventory (DAI), as a relatively comprehensive system of categories. I called the behavioral norms contained therein *actual capacities*, an expression I held to be necessary because these norms are inherent as capacities in human development; they are the developmental dimensions, the shaping of which is furthered or suppressed by favorable or inhibitory environmental influences" [7].

The *modelling dimensions* of Peseschkian described later are also close to the humanistic approach: "As a result of interaction with the environment, and particularly as a result of evolutional interaction with others, the structure of the self is formed—an organized, fluid but consistent conceptual pattern of perceptions of characteristics and relationships of the 'I' or the 'self,' together with values attached to these concepts."

This terminology and method of Peseschkian are in line with the early phase of humanistic psychology. The difference from this approach is reflected in a structured approach, in the application of a semi-structured first interview and a five-step process in therapy, family, and self-help, in the use of visualizations such as the balance model, modeling dimensions, stories [11], proverbs, and questionnaires for the actual capacities and conflict reactions (see also Chap. 2 on the DAI and the Wiesbaden Inventory for Positive Psychotherapy and Family Therapy [WIPPF] questionnaire). The therapeutic process was seen in a five-step development. In this aspect, PPT is also close to the organized way Alfred Adler conducted his individual psychology therapy process in five steps and a sixth step of self-help. Alfred Adler's method seems to play an important role for Peseschkian to combine the psychodynamic and humanistic background of PPT and its process with the application of family styles, self-help, encouragement or actual capacities as psychodynamic conflict content.

Psychodynamic Theory of Conflict, Personality, Relation, and Symptom

Meetings and training took place with psychoanalysts in Frankfurt, Germany, where Peseschkian worked in the University Mental Health Hospital. He learned from well-known Swiss psychoanalysts such as Heinrich Meng in Basel, conducting his psychoanalytic self-experience, from Raymond Battegay, who was like a mentor and

friend for him, and from Gaetano Benedetti. Their influences encouraged Peseschkian to develop a psychodynamic conflict theory, fitting the needs for a clear psychodynamic diagnosis in the Germany of the 1970s.

In connection with the psychosocial norms, [Peseschkian] asked [himself] the following questions: How do conflicts arise? How can these conflicts be adequately described? What lies behind the symptoms of psychological and psychosomatic disturbances and the curtailment of interpersonal relationships, and how can these disturbances be adequately treated? (1987). He described the resulting subconscious inner conflicts as following a partly conscious actual conflict situation, waking up former basic conflicts.

From 1968 on, Peseschkian developed the "differentiation analysis" or "differentiation-analytic theory" [7] and published it at conferences in 1974 as a complement to the psychoanalysis of that time, which was concerned primarily with the psychosexual phases of development (for example, oral, anal, and Oedipal), the development of autonomy and conflicts between the Id and the superego. In differentiation analysis, the client and the therapist work out which specific contents and values have been important at earlier stages, and which are still important today: the patience of the parents, the development of trust, the experience of love in unconditional acceptance is a developmental psychological prerequisite for successful development in the oral phase. These capacities, known as "primary," are imprinted on the child by the direct behavior of the parents and through their modeling. Primary capacities such as having patience (with oneself or others), having trust (in oneself, in others, or in fate), having and giving time, are basic necessities for the development of the newborn child. The child needs warmth, time, patience, and empathetic, unconditional acceptance to develop her/his own age-appropriate inner balance. In this way, the phases of psychoanalytic development psychology had been filled with specific terms for conflict content defined as capacities, understandable by everybody, linked to cultural values.

The experiences of the clients with their parents, and with other reference persons, and their values, capacities, and social norms are described within *four* role *model dimensions*. These occur through the child's direct relationship with the parents, through experiencing the relationship of the reference persons with one another, with others, and with their philosophy of life. The actual symptom of a patient can therefore be understood on the one hand as the expression of an inner psychic conflict situation, and on the other hand as being culturally imprinted and as receiving its social function from within the patient's social system. This is also true for the capacity to bond and relate, the early development of trust, as opposed to mistrust, the basic attitude toward life, identity, and self-image. Above all, the primary actual capacities are developed in this area in mutual interactions that demonstrate the modeling functions of the primary reference person. This is equivalent to the experience of the self-object and the development of self- and object representation in psychoanalytical theory. The grandparents are also important here in their independent role as trans-generational transmitters of tradition and preferred primary capacities, and in the widening triangulation, they play a significant role in their grandchild's development of self-esteem because of their special form of acceptance of him or her ([12], p. 126). With the role model dimensions Peseschkian widens the self-object theory of Kohut and Kernberg to apply this wider concept of psychodynamics and sociodynamics to collectivistic and family-oriented cultures as well. In this way, Peseschkian became a proponent of a bridge between individual psychotherapy theories and the collectivistic cultural needs in treatment and counseling.

The individual psychodynamic conflict model and the psychogenesis are complemented by a model of *sociodynamics* and *sociogenesis* [13]—and this is a remarkable addition to the individually oriented psychodynamic therapy. Jacob Levy Moreno was one of Peseschkian's teachers and the inventor of sociodrama and sociometria. Peseschkian coined the terms sociogenesis and sociodynamics to describe the dynamic and gen-

esis of the family and cultural value system and concepts within the terms of actual capacities. In this way, it became a bridge between the systemic and humanistic and the psychodynamic understanding. Along with the intrapsychic conditions of the formation of health or disturbances and of the individual psychodynamics, it is the family, social, cultural, and transcendental relations that imprint the concepts of sociogenesis and the value models, forming a culturally imprinted sociodynamic.

Peseschkian's model also leaves the classical deficit, instinct, and pathology orientation in psychoanalysis and comes back to the "potentials" in the clients, another humanistic term used by authors such as Maslow: "Suppressed and one-sidedly unfolded capacities are possible sources of conflicts and disturbances in the psychological and interpersonal areas. They may manifest themselves in anxiety, aggression, conspicuous behavior, depression, and that which is called psychosomatic disturbance. Since the conflicts arise in the course of a person's development in the confrontation with his environment, they … present themselves as problems and tasks which we seek to resolve. With this, an essential difference becomes clear: traditional psychiatry and psychotherapy take as their point of departure disturbances, conflicts, and illnesses. Accordingly, the goal of treatment is set: to heal illnesses and eliminate disturbances. What is overlooked is that it is not disturbances which are primary, but rather capacities, which are indirectly or directly affected by these disturbances" (1987).

As with other approaches based in "depth-psychology", "Tiefenpsychologie" as psychodynamic short-term therapies are called in Germany, PPT recognizes a psychodynamic conflict causative agent. This is, however, more sharply differentiated and complemented by the theory of *microtrauma*. It means a repeated subconscious conflict of values that will not be existential each time, but in its sum the balance and inner conflict resolution capacities of a person will be disturbed, so that symptoms can be the subconscious answer and language of body and soul on the repeated smaller conflicts touching old basic concepts and basic conflicts. "Conflict" (derived from Latin *confligere* meaning clash, fight) presents the apparent incompatibility of inner and outer values and concepts or an inner ambivalence. Emotions, affects, and physical reactions can be understood in this connection as signal lamps that indicate an inner conflict of values and the distribution of actual capacities. Therefore, in PPT the question is asked about the content: Exactly what triggers this emotion? The conflict reactions, defense mechanisms, and resistance are then seen as *capacities* or *capabilites* and viewed according to their functions.

The *balance model* broadens Freud's concept of the "libido," as with Adler, to four areas of a general and pro-social life: energy of body, activity, social drive, and existential or spiritual motivation. Areas of conflict reactions can be specified in the four areas in their influence on actual everyday life, in their function and spectrum of symptoms.

Interactions with others are described by the *three stages of interaction* on the basis of a specific developmental psychology. Natural stages of human interaction occur in the traditions of greeting, meeting, and separating. "Hello!" shows the attachment. The question that so often follows: "How are you?" introduces the differentiation as well as the exchange of news. The "Good-bye!" is the sign of detachment, combined with the wish to meet again. Thus, the three stages of interaction in PPT can be understood as three phases of a meeting in which the orientation of one toward the other and expectation for the future play a role. Affect and emotions are seen as outward expressions of content behind which are found values and value conflicts (*actual capacities*). Relationship models are described according to the three stages of interaction and explored for any one-sidedness that may lie behind them. In one's individual development as in a situation of partnership, one moves continuously through different stages, which are characterized by the following principles: the principle of development, the principle of differentiation, and the principle of unity. In human relationships these principles are equivalent to the three stages

of interaction: fusion (attachment), differentiation, and breakaway (detachment, separation), which are observable in every conflict and in every meeting [7]. They structure the life of all human society [11].

Within psychodynamic theories, the three stages of interaction represent a broadening of the duality of dependence and autonomy in relation to differentiation. They are at first glance similar to the three levels of meaning in medical treatment described by Victor von Gebsattel, the stage of being summoned in an emergency, the distancing of the diagnosis, and the coming together as partners in the treatment [14]. As Viktor Frankl and von Gebsattel were in close contact, and Nossrat Peseschkian was a student of Viktor Frankl, it is probable that von Gebsattel's three phases of the medical encounter had some influence on the reflections that led to the stages of interaction. However, Nossrat Peseschkian based the stages of interaction on the history of a child's development, then carried them over into the therapeutic process, the general patterns of human interaction, and the analyses of interaction described above.

A longtime companion and friend of Nossrat Peseschkian's, the Swiss psychoanalyst Raymond Battegay describes PPT in this connection as: "…a method of depth psychology which enlists not only the individual unconscious but also the collective/typical in the clarification of the psychodynamic. The unconscious is understood by means of transcultural comparison even more than in the junction of analytical or complex psychology."

In this way, PPT is clearly defined as based on a *psychodynamic conflict theory*. The *basic conflict* of the early experience in childhood uses Freud's term, and the *inner conflict* is defined as being close to modern psychodynamic therapies. However, the inner and the basic conflict exist between the needs of primary capacities and the social norms represented in secondary capacities [15]. In this way, the clients can easily understand their formerly subconscious *conflict dynamic* and *conflict genesis* in terms of actual capacities such as trust, honesty, justice, orderliness, or hope.

Transcultural Psychotherapy: The Fourth Way

In 1955, a program in transcultural psychiatry was established at McGill University in Montreal by Eric Wittkower, from psychiatry, and Jacob Fried from the department of anthropology. The American Psychiatric Association established a Committee on Transcultural Psychiatry in 1964, followed by the Canadian Psychiatric Association in 1967.

Transcultural psychiatry had been working with the differences between mental health disorders in different countries, but a transcultural psychotherapeutic treatment had not yet been developed. It seems that Nossrat Peseschkian consecutively invented his "transcultural psychotherapy" in 1968, and from 1979 on, he used the term "transcultural psychotherapy" for his approach. As a practitioner, Peseschkian not only looked at the intercultural perspective but also at the transcultural way: "Transculturalism is defined 'seeing oneself in the other'. Transcultural … is in turn described as 'extending through all human cultures' or 'involving, encompassing, or combining elements of more than one culture.'" "In 1940, transculturalism was originally defined by Fernando Ortiz… as the synthesis of two phases occurring simultaneously, one being a deculturalization of the past with a métissage … with the present, which further means the 'reinventing of the new common culture.' According to Lamberto Tassinari transculturalism is a new form of humanism based on the idea of relinquishing the strong traditional identities and cultures which […] were [the] products of imperialistic empires […] interspersed with dogmatic religious values [16, 17]." Thus, we are back to the humanistic approach that made transcultural psychotherapy possible. In it, the transfer of the knowledge and the wisdom of different cultures in therapy and counseling as "seeing oneself in the other" is the new approach in therapy and seems to be one of the reasons why PPT is accepted in many different cultures and countries.

Culture and religion are consciously presented as themes in PPT. This is similar to the use of these themes in the logotherapy of Viktor Frankl, one of Peseschkian's teachers. The idea of a collective unconscious, described by C. G. Jung, with its cultural symbols can be seen as a basis for a transcultural understanding of meaningfulness, religion and spirituality, Both C.G. Jung and Nossrat Peseschkian take as their point of departure a person's particular experience of transcultural and religious conflicts, and an entirely personal motivation to find solutions. Consequently, in both models, transcultural observations and deeper experiences occur at the end of the therapy and the patients' personal development stands at the center of the process. As they were both empiricists and transcultural pragmatists, they have brought visualization into psychotherapy. Using stories, concepts or symbols, dreams or images that have been passed down, they create from the common treasure of humanity. Both methods have systems for personality typing, four areas of distribution of personal energy, two forms of expression of conflicts, and of working with them, in addition to the influence of the collective unconscious in relation to the cultural influence found in personal development. However, the consequences of their models of development, in addition to the theories derived from them, differ considerably.

According to Nossrat Peseschkian, the capacity to love is equivalent to the emotional content of an interaction, as with patience, time, love, sexuality, modeling, and trust. The capacity to know constitutes the social norms, such as punctuality, orderliness, obedience, diligence, faithfulness, or justice. These characteristics as such are found in all cultures. They differ from one individual or culture to another in their valuation. In addition, there is content from the inheritance of human history, which, according to Jung, finds its expression in symbolic images from the area of the collective unconscious. It is these symbol-laden images that Nossrat Peseschkian's therapy explores for their content and socio-cultural valuation.

Cognitive and Behavioral Therapy

Sometimes it is stated that Peseschkian helped to build bridges among behavioral therapy, psychodynamic therapy, and other therapies. Are there roots of PPT in behavioral therapy? Hans Deidenbach, a former monk, as a psychologist and as one of Peseschkian's most active co-workers from 1972 onward, had been trained as a behavioral therapist. His influence in the development of PPT and the publications was evident in that it was described systematically, resulting in the WIPPF questionnaire, and in the semi-structured first interview (1988). Concerning Dieter Schön, another early medical co-worker of Peseschkian since 1968 and contributor to his articles, discussions with Lilo Süllwold, a professor from Frankfurt, behavioral therapist and author of the FBS and FBB questionnaires led to a common understanding and integrative ideas for both methods. Peseschkian himself described the relationship with behavioral therapy in the chapters of "Positive Psychotherapy and Other Theories" ([7], p. 375).

Cognitive behavioral therapy was published by Aaron Beck in 1976 [18]. The descriptions of "concept" (Peseschkian) and "cognition" [19] (Beck 1990) are quite similar. Peseschkian's "concepts," based on "actual capacities," as "virtues" or "values," are comparable with "cognitions" in cognitive behavioral therapy [19, 20]. They are a bridge to making cognitive behavioral therapy understandable for clients and therapists. However, PPT developed these capacities and concepts, independent of cognitive behavioral therapy, on a humanistic and psychodynamic basis. PPT became an integrative tool for translating other psychotherapeutic methods into a language clients and therapists can understand.

Positive Psychology

Positive psychotherapy after Peseschkian [7] and positive psychology after Martin Seligman [20] differ with regard to the structure of the treatment process, and to their theoretical background. The

orientation toward salutogenesis and psychodynamics in PPT has in mind the inner subconscious conflicts causing symptoms, whereas Seligman's approach is based on learned helplessness and learned happiness. In a study of depressive mood disorders with 40 students and 46 depressive persons, Seligman and his colleagues used "positive psychotherapy" as their own term [22], applying virtues training. The strengthening of positive emotions figures significantly in this treatment (the opposite of Seligman's theory of learned helplessness), in addition to striving toward engagement and meaning in life by using techniques from positive psychology, rather than dealing with the depressive symptoms directly. Seligman proposes to lay out the meaning of happiness in three scientifically verifiable terms: positive emotion (the pleasant life), engagement (the engaged life), and meaning (the meaningful life). "Each exercise in [PPT] is designed to further one or more of these" ([22], p. 776). Exercises and methods from behavioral therapy are employed and 24 strengths of character and virtues are differentiated. The facilitation of orientation, engagement, joy, and the demonstration of positive resources are steps in this therapy.

Seligman's view seems to be similar to that of Peseschkian at first sight, and quite different when looking at the psychodynamic and humanistic therapeutic approach integrated by Peseschkian. Psychotherapy has been entirely taken up with trying to repair the negative for so long, now it is the time to move on to the positive. Unfortunately, Peseschkian, whose book "Positive Psychotherapy" had already been published in English in 1987 and available in the libraries of American universities, has never been cited in this context.

The concepts of positive psychotherapy used by Peseschkian as a treatment method in mental health, family therapy, and psychosomatics were first used in Europe, and were published worldwide in the 1970s [7, 12]. A co-worker of Martin Seligman's, C.R. Snyder, author of the "Handbook of Hope" (2001) was invited to the Second World Conference of Positive Psychotherapy in Wiesbaden (Germany) in 2000. He was then exposed to the theory, practice, and international spread of PPT, which he took back to the USA. The German Medical Journal (*Deutsches Ärzteblatt*) published an article in which both methods were presented side by side, and Christian Henrichs commented on it later [23]. Theo Cope compared the development and background of PPT and Seligman's positive psychology [24]. Seligman and Rashid had used the term "Positive Psychotherapy" since 2006 without mentioning Peseschkian, and Rashid took part in a conference of the World Association of Positive Psychotherapy in Kemer/Turkey in 2014 and also in a round-table discussion about the two methods. Peseschkian's PPT had been developed since 1968, published in 1977, and applied as a transcultural, mental health, and psychosomatic treatment method against a humanistic and psychodynamic background with a structured therapy process to treat ill patients and families. Positive psychology uses its interventions for healthy humans against a psychological background of behavior theory and research about learned helplessness and learned happiness, today with the goal of flourishing and virtue development [25].

Family Therapy

Family therapy was developed mainly in the USA, Italy, and Germany at the same time as Peseschkian founded PPT and differentiation analysis. It was a developmental period for different kinds of systemic family therapy, such as Maria Selvini Palazzoli, also using positive connotations, the Palo Alto school, or Virginia Satyr. Peseschkian's idea was to combine individual therapy with partnership and family therapy, applicable both in collectivistic and individualistic cultures. He understood the symptom in its function within the family and used family therapy tools not only for a single member with regard to how to self-help but also for the complete family to see the symptom as an expression of a conflict in the family system. Different from

systemic therapy and unique in Peseschkian's therapy are the instruments for self-help in family therapy, such as differentiation analyses, WIPPF, balance model, role model dimensions, first interview structure, or the five-step self-help concepts [26, 27].

Self-Help, Pedagogy, and Process Orientation in Five Steps

Alfred Adler and Nossrat Peseschkian had one thing in common: they both left their practices early on in order to share their knowledge of psychotherapy and education with the general public. Both focused on public health, medical and psychological prevention, and social welfare, both wanted each client to have an own individual type of therapy. Both founded a five-step process model for communication, therapy, and self-help. The basic structure of individual therapy in Adlerian psychotherapy consists of five phases and a follow-up. Each of these stages has different tasks for the client and therapist.

In the first stage, the therapist provides warmth, acceptance, and generates hope, while giving reassurance and encouragement to the client in Adler's approach as well as Peseschkian's. The second stage in the Adler method is to focus on gathering information on the client, like Peseschkian's inventory. Adler used clarification and encouragement similar to Peseschkian's situational encouragement in the next step, followed by Adler's insight interpretation and recognition, a stage like Peseschkian's verbalization, and finally the change and challenge steps are similar to Peseschkian's widening of the goals. It seems that both thought in a very similar way about therapy and education; thus, it is understandable that Adler's and Peseschkian's methods are useful today for parents, in self-help, and in education.

The five steps concept seems to be like a natural law for groups and communication, as Moreno described five phases of psychodrama, Battegay five phases of the group therapy process, and Adler five phases of therapy and one more for self-help. Peseschkian's ideas about these five steps formed a systematic and simple skill for the communication process.

The Construction of PPT in Comparison with Other Approaches

1. The *positive approach* of Peseschkian is very similar to Selvini Palazzoli's family therapy practice at the same time, and very similar to what the German transcultural psychiatrist Mentzos later saw as the "function of dysfunctionality" [28]. Similarly, Geoffrey Zeig explained that not only the "individual capacities of the client but also the problems of the clients can be utilized to construct solutions out of this." The positive approach was used in psychosomatic medicine by Peseschkian very early on.
2. The *transcultural approach* compares concepts with those from other cultures. Stories from other cultures are used, like oriental stories in Western culture, to surprise people with another cultural perspective. In this way, it is similar to hypnotherapy, where for instance Milton Erickson also used stories. Peseschkian clearly defined a transcultural approach to using stories and proverbs to change the point of view of the client, to see his/her own situation and the function of the symptoms in a new way.
3. The *first interview* after Nossrat Peseschkian, a semi-structured interview with the client, developed with his coworkers Hans Deidenbach and Hamid Peseschkian, systematically gathers all the information for the client and the therapist to plan an individualized therapy. Hamid Peseschkian's dissertation [32], presented in 1987, was the first doctoral dissertation dealing with PPT. The first interview in PPT was first structured in this dissertation, a questionnaire for this first interview was presented, and a psychodynamic study of it was undertaken. This first interview questionnaire was published

shortly thereafter in 1988 (with minor modifications) together with the WIPPF questionnaire on PPT.

4. The *balance model* is used for therapy, self-help, family therapy, psychosomatic, and counselling. This balance model of the four areas of life energy broadens Freud's libido construction, involves Adler's goals of life, and includes Jung's four functions (perception, ratio, sensitivity, intuition). It shows possibilities for a new balance of those areas that were in deficit; therefore, a new synthesis can be arrived at within the framework of the therapy.

   The operationalized psychodynamic diagnosis (OPD) describes four observable basis capacities of the personality structure, which are quite similar to the four means of the capacity to know [7]:

   (a) The "means of the senses" (Peseschkian) is comparable with the structural capacity to perceive oneself and others, as described in the OPD. In the foreground, there stands the body—the self—the feeling ([11], p. 94).
   (b) The "means of reason" serves in Peseschkian's balance model as the reality check through which problems can be resolved systematically ([11], p. 96) and it directs our activities. It is related in OPD to the structural capacities that regulate our inner and outer impulses.
   (c) The "means of tradition" serves as the capacity to start and to communicate in relationships ([11], p. 97). In OPD, the analogy to this is the emotional communication with oneself (internal dialogue) and with others as the structural capacity for empathy, and the anticipation of relating to and thinking about others.
   (d) The "means of intuition" is described by Nossrat Peseschkian as the fourth area, that of meaning, future, and fantasy. He defines it as the capacity to imagine something in one's mind. Intuition and imagination can go beyond immediate reality and take in whatever we can depict as the meaning of an action, the meaning of life, desires, pictures of the future or utopia ([11], p. 99), ideals, or models. In the OPD, the fourth capacity of the personality structure is the "capacity to attach to internal and external objects" [21]. Included in this is the imagination of objects that provide support, the connection to an ideal, and the external connections with persons. In Nossrat Peseschkian's scheme we understand this to be the capacity for imagination. With its help, the small child can imagine its parents or emotionally close persons, so that after a period of being alone, the imagination of these persons can quieten down, in contrast to those with structural disturbances, who often find such an image impossible to imagine. In this sense, the distribution of the four areas of the balance model represented a forerunner of the structural models in the OPD-2 [21] in 2006.

5. *Differentiation analyses* defining capabilities involved in concepts and conflicts are based on the idea of developmental potentials in humanistic psychology, as well as on the conflict model of psychodynamic therapy. It is a link between humanistic and psychodynamic therapy with terms easy to understand by the clients.
6. The two *basic capacities* – to know and to love – are quite similar to the humanistic approach and are based on Peseschkian's personal experience of the Baha'i Faith terminology.
7. The *three stages of interaction* represent a broadening of the duality of dependence and autonomy in relation to differentiation, a further development of a description of the medical doctor–patient encounter by Gebsattel at the levels of being summoned in an emergency, the distancing of the diagnosis, and the coming together as partners in the treatment [14]. Nossrat Peseschkian based the stages of interaction finally on the history of a child's development, then carried

them over into the therapeutic process, the general patterns of human interaction, and the analyses of interaction described above. The result was the invention of an interaction analysis specific to PPT in addition to the application of the three stages of interaction in a therapeutic relationship, in partnership counseling, and in children and youth therapy.

8. The four role *model dimensions* widen the analytic self and object theories of Kohut and Kernberg in such a way that there is not only the object and subject, the "I" dimension, in PPT but also the parents' partnership, the "You" dimension, a "We" dimension such as the experience of the parents with others, and the "Primary We," describing the relations of the primary important persons such as the parents and grandparents with their life philosophy and religious backgrounds. With these four different subject relations, the self-object theory is widened, and helps to discover the basic conflict contents in psychodynamic therapies more easily.
9. Language, stories, and proverbs offer a *narrative and associative approach* [31]. In psychodynamic therapy, this was unique in the 1970s, it had existed in hypnotherapy (Milton Erickson), but not in psychoanalytic therapy. While Carl Gustav Jung used fairy tales, the Peseschkian method uses a wide range of narrative therapy and association tools. "Using *stories and parables* from the Orient and other cultures, an effort is made to recognize and further a person's potential for self-help. With reference to the symbolic meaning of proverbs and old words of wisdom drawn from many cultures, the person to whom they are told is led in psychotherapy to a more positive view of himself" ([29], p. 92).
10. The *five-step concepts* of individual and family therapy—like five fingers of each hand—are quite similar to the natural process in groups as described by Raymond Battegay in group psychotherapy, as described by Moreno in psychodrama, and as described by Alfred Adler, using it for the "further education" of people. The unique contribution in Peseschkian's use is that this process model now exists in psychodynamic therapy. The five steps are a guideline for the client and the therapist to finally find the right way to self-help. Therapy research showed that the better we cope with difficult therapy situations and the better we both reflect the therapeutic relation in each session, the better the outcome of therapy will be.

Conclusion: The Place of PPT in Psychotherapy Fields

The roots of PPT are to be found in humanistic psychology and in psychodynamic therapy, seen from the scientific point of view. There are other roots to find in the religious life philosophy of the founder, as Theo Cope described in 2009 ([30], pp. 79–89). The constructions of PPT are based in scientific theories that exist nowadays in other therapies as well. There was a unique development of Nossrat Peseschkian's method, widening psychodynamic and humanistic psychotherapy theories and practice, creating a transcultural psychotherapy and implementing an integrating approach, concerning the individual needs of the client, salutogenesis, and family therapy and self-help tools.

Positive psychotherapy was originally developed for mental health, psychosomatic medicine, prevention, and psychotherapy as a basic *positive psychosomatic* treatment, as used by several thousand medical doctors in Germany. It is applied in hospitals, in outpatient treatment, and part of the state-licensed training program in psychodynamic therapy at the Wiesbaden Academy of Psychotherapy in Germany.

PPT as an integrating method has also been applied for counselling in Germany since 1992, and in some countries such as Bulgaria in pedagogy as well since 1992. In China social workers have been trained since 2014 about positive work with mental health disorders, how to encourage the families, and how to prevent burnout. PPT is an example of how to apply psychotherapeutic competences outside the classical psychotherapy and counselling fields. A specialized children and

youth therapy training program, based on PPT has been developed since 2006 in Bulgaria and later in Ukraine and Russia. Colleagues from Germany, Bulgaria, Cyprus, Turkey, Kosovo, China, Bolivia, and Ukraine are specializing in positive family therapy and counselling. The method became a bridge for sharing psychotherapeutic competences and experiences in other professional and cultural fields across different societies also as a link between different psychotherapeutic theories and approaches.

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The First Interview in Positive Psychotherapy

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Arno Remmers and Hamid Peseschkian

Introduction

The semi-structured "First Interview of PPT" allows clients and therapists to recognize the particular psychodynamic and biographical characteristics and resources important to the origin and development of the patient's state of health or illness. It is a systematic starting point for brief therapy. Described first by Nossrat Peseschkian [1], it was structured by Hamid Peseschkian in his dissertation [2]. Combined with the Wiesbaden Inventory for Positive Psychotherapy and Family Therapy (WIPPF) questionnaire, a patient's therapy calendar (timeline), and a therapist's calendar, it was published as First Interview Questionnaire [3] (see Appendix A for the complete form). It is comparable to the medical history and physical examination in somatic medicine. Previous work on the first encounter included "First Interview" by Argelander [4] and "The Psychodynamic First Interview" of A. Dührssen [5]. Further investigation found the first interview had a diagnostic, therapeutic, and prognostic value and a hypothesis-formulating function [6]. The First Interview in PPT includes all of these as well as several other elements. It cares about the psychosomatic medical history and about relationship factors [7] as well as aspects of the therapeutic alliance (Hubble et al. 2001). It includes the recognition of the effects of expectation [7], particularly the hope for effective therapy [8, 9]. All this is useful in different settings; in therapy with individuals, children, youth, couples, and families; and in counseling and coaching and applied in different cultures.

Structure

The First Interview in PPT consists of obligatory main questions and optional subordinate questions, which are optional, depending on the answers given to the main questions in each situation. There are both open-ended and closed questions [2].

The First Interview is used in the first encounter or during the early sessions which form the preliminary phase for the purposes of orientation and then goes into greater depth in particular areas during the following sessions.

The First Interview is divided into four areas:

1. Socio-demographic data.
2. Current history and main complaints.
3. Psychosocial situation.
   - 3.1. Actual conflict.
     - 3.1.1. Macrotrauma: Stressful life events during the last few years.

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_34

3.1.2. Microtrauma: Contents of the daily conflicts (actual capacities).
3.2. Balance model: The patient's present life situation.
3.3. Basic conflict: Childhood experiences and concepts development.
4. Diagnostic reflections and further procedures.

The structure given above is further embedded in the three stages of interaction of PPT: attachment, differentiation, and detachment [10]. In the context of the First Interview, *attachment* means the patient's developmental relationships and his ability to observe the scene around him. It goes on to include diverse questions, in the context of a therapeutic process, which help open up the atmosphere and form the basis for finding out about existing trust and hope within the patient. In the second stage, *differentiation*, information relating to the patient's psychosocial situation and values is collected. The third stage, *detachment*, deals with explaining the diagnosis to the patient and the discussion of further procedures (treatment plan). The patient's hope is strengthened through changing the point of view by the use of stories, proverbs, and figures of speech. The basis of the therapeutic relationship is established, and an informed consent is agreed with the patient about the tasks and ways of therapy.

The *socio-demographic data* which are produced are common objective data that can unlock the origins of current stressful life events and subjective experience. In practice this has another important function. During the first sessions, the therapist can take in the most important data in a short form (age, family situation, professional ability, parents, brothers, sisters, etc.) in order to place the patient's subjective experiences into their context. An ordinary patient file can be enhanced by the recording of additional objective data (education, partner relationships, etc.). Recent data are important for the building of a report for an expertly crafted case. However, most of this cannot be gotten within the time of the first session. Also, many patients experience being asked for objective data at the beginning as a great unburdening, and it is only after this that they can present their subjective suffering.

The *actual history* begins with the patient's current subjective complaints and problems, which caused him to seek a therapist. Even at this early stage, the therapist, in an effort to discover the function of the symptoms, named "positive connotation" by Nossrat Peseschkian, tries to find an initial possible explanation for them, for example, the fear of loneliness as the need for contact with other people or that a psychosomatic complaint is the somatic manifestation of a psychic conflict, such as telling a patient with severe eczema that "something has gotten under your skin" or that a hearing loss means not to have to hear something or someone any longer. The development of psychodynamics can be compared to a puzzle whose pieces must be searched for in the deep psychological exploration of the subconscious [11]. The symbolic function of the disturbance is one of the first and most important pieces of the puzzle. To put it simply, one can say: Patients can hide something or lie, but not their symptoms. We gain understanding of the symptoms, the time when they began, and their function as catalysts for understanding significant inner conflicts. This section contains questions about previous illnesses, treatments, and risk factors.

After dealing with socio-demographic data and current complaints, we come to the questions about *stressful life events*, a natural transition for both patient and therapist. The opening question in this section of the First Interview in PPT reads:

> What has happened to you and your family during the last few years?

Most of these significant life events (separation, job loss, illness, death of important persons, etc. and also positively connotated events like giving birth, marriage, or new jobs) were already mentioned earlier in the section dealing with the biographical data, but now they are explored in more detail and in different areas of life. These events are noted down in the balance model by the therapist in order to explore them more specifically later. Then the events in all of the areas of life can

be asked about and dealt with. The authors like to ask in this context about the influence of the illness or problem on the patient and his environment in the four areas of life:

> What influence has your illness had on your physical well-being, on your work, on your family, and on your view of the future?

This is also where the impact which the present problem has had on the patient can be clearly seen. The preparations for exploring this can be made either now or in later sessions. However, what is essential in the first session are the sociodemographic data, the symptoms of the actual complaint, and the questions about stressful life events, and the ways of coping. Questions about the current life situation in the four areas of the balance model, the distribution of energy, and the significance of specific areas of life on the patient's self-esteem are connected, followed by those relating to the influence his illness has had on the four areas of life. By means of this graphic presentation, the patient begins to recognize the one-sidedness of his way of life on the one hand and his possibilities for development within the framework of therapy on the other. Visualization is an important instrument in PPT for patients to create an own model of understanding of their situation and symptoms, to find a symbol or narrative for their own life.

One distinguishing feature of PPT is the description and understanding of imprinting situations in early childhood which found basic concepts and can lead to a basic conflict. In the First Interview, the patient is asked questions within the context of the four modelling dimensions about his relationship to his parents, their relationship with one another, their relationship to society, and their value system. Even though this sometimes causes some surprise in our psychoanalytic colleagues—why would one wish to identify the basic conflict so quickly—these questions grasp very well the early childhood situation and its influence on the present concepts and capacities, though they are presented briefly and graphically. Of course, these themes will be explored at greater depth during the psychotherapy itself. These questions which illuminate the basic situation form the basis on which a primary psychodynamics hypothesis can be formulated. Now PPT provides the explanation for the patient. He is asked to summarize what was most important for him in this meeting and what conclusions they suggest about his capacity for reflection. The therapist also offers some summarizing observations, his view of the assessment of the main symptoms, of possible basic conflicts, and of their actual manifestations. This is an important component of the questions asked by every patient: How to proceed from here, what is going on with me, and is there any hope that I will be able to get out of this situation? Depending on the patient, it may be possible to give him a task to do at home and bring next time, to write the details of these stressful life events, or to discuss the discovered subjects at home. If applicable, the patient can be given the story "On the Way" [12], as an associative tool for his or her own situation. These approaches prepare in an early stage self-help and self-exploration by the patient to become independent of therapy as early as possible.

Nossrat Peseschkian's First Interview responds to the perception of objective and subjective data just as Argelander [4] had put this data in the forefront. The "scenic description" of Argelander, as an impression of the encounter scene, will be described in PPT using additional terms like the actual capacities and describing the type of interaction pattern. The psychodynamic first interview [5] raises the particular significance of the social relationship structure and family which are systematically presented together here in the First Interview in PPT. Therapy can be planned with it in which the social resources can be purposefully collected and considered. The model presented here clarifies factors and themes which are functioning for a psychodynamic treatment plan. The *Operationalized Psychodynamic Diagnosis* [13] defines conflict contents, relationship models, and structural elements of the personality which are constantly coming up again in current relationships and predominate in relationship episodes. In the First Interview in PPT, these dimensions are more sharply differentiated

according to their contents. Their historical sense is defined, and the history of the relationships is traced back as well as connected with the background of family concepts. The use of this procedure in the guidelines for psychodynamic therapy was described in the book *The First Interview* [14].

Therapeutic Attitude and Competencies Needed in First Interview and Therapy

A variety of different behavioural patterns are necessary in the first interview and in psychotherapy: i.e., the supporting aid in a deeply depressive patient, the structuring in affective disorders, the freedom of association after telling a story or metaphor, surprising, or postponing subjects. A whole variation of accepting, supporting, surprising, complementary, and contrary behavior seems most effective in client care, both rationally and emotionally.

Grawe [15] and others showed the effectiveness in psychotherapy of the following factors: aid in conflict solving, understanding and clearing, quality of the therapeutic relation, optimism to find and to give help on the outcome of psychotherapy, and personality and maturity of the therapist independent of the method he uses.

Competencies required for a comprehensive first interview [10]:

1. *Observation—Distancing*: The ability to listen patiently with empathy, to accept and understand, and to add other points of view. In PPT, therapists need their capacity to be sensitive to the client, looking first at the capacities, to see the functions of the symptom using a positive connotation, or to compare transculturally with other points of view, to visualize, and to find the language the client can accept while looking like from a distance on oneself, changing of point of view.
2. *Inventory*: The ability to ask exactly and to define contents, capacities, history, areas of conflicts, and conflict reactions of the client.
3. *Situational encouragement*: To encourage the self-aid of the patient (support, self-help, and the patient as therapist for his environment), resources are developed; the patient and his surroundings play an active part in the understanding of the process of illness and self-help in this step.
4. *Verbalization*: The ability to solve problems in frank consultation limited to the area of the problem; the conflict is subsequently worked through in its ambivalence having in mind different ways and consequences; and to see responsibilities for the four areas of relation. Included is the interactive experience of the key conflict courtesy and openness and actual capacities within the therapeutic relation as a field for training.
5. *Broadening of goals*: The ability to ask about the future after working out the conflict: to ask what the patient will be healed for. It is the step to see conflicts and the feedback about the treatment as an opportunity which includes looking back, summarizing, and testing the new concepts and strategies and new perspectives.

This therapeutic process as a whole is predominantly oriented toward the future and toward change, and it involves understanding the past through concepts which are effective for the current time. In the therapeutic alliance, the process is reflected and checked, which phase is just going on, and to initiate the next phase by questions. The goal is to start each session with the first step in attachment and to end up with feedback and perspective as a last step. The pace of moving through steps can vary according to the flow of the conversation. The same process is applied in self-experience (personal therapy) of therapists in their training and in supervision.

The Five Stages of the First Interview Process in Practice

The five stages of therapy provide a structure for the process of communication within a single session or during the whole therapeutic process, which would spontaneously come to an end without it. The therapist facilitates them by means of

an appropriate attitude, open questions, stories, association triggers, and re-visiting themes previously mentioned. The five-stage process in PPT provides both the therapist and the patient with a starting point as well as a sense of security and prepares the patient for the work with conflicts and for self-help, particularly for the time after the end of therapy.

Phase of Observation and Distancing

The actual history begins with the patient's current subjective complaints and problems, those which caused him to seek a therapist. The therapist, in an effort to discover the function of the symptoms, will gain an understanding of the symptoms, the time when they began, and their function as catalysts for understanding significant inner conflicts. This section contains questions about previous illnesses, treatments, and risk factors. Disturbances and illnesses are seen as capacities which react in conflict, the function of the illness, its meaning, and consequently its positive aspects [16].

Therapists can start with the inner attitude like "Be yourself here and now in the therapeutic relation with the client, be sensitive and open for changing the point of view on the client and her or his uniqueness." This is the potential orientated humanistic approach with acceptance, an honest and sensitive relation similar to the conditions Rogers described. To look at the clients' capacities in every description, therapy situation with each other or with others, the possibility of development and change, helps to change the view to a positive interpretation of the whole situation and the symptoms. The meaning of the term capacity is close to the ideas of development of the person described by Maslow as the importance of focusing on the positive qualities in people, as opposed to treating them as a "bag of symptoms" (see Chap. 33).

The patient begins with an emotional supposition, symptom description, and impact on life. He is led from an abstract stage of suffering to a concrete, descriptive point of view. The patient is brought to an understanding of the functions and effects of his symptoms in the four areas of life, with the use of figures of speech and transcultural comparisons. As part of the therapy, the patient is asked to observe the situations he or she experiences and his or her own emotions, particularly with conflicts, and to write it down as spontaneously as possible without changing anything. Viewing his own conflicts from the position of an observer helps the patient attain a growing distance from his own conflict situation. He becomes an observer of himself and his environment. An important effect of this stage is the high level of unburdening in a conflict situation.

In the same way, the stage of observation is helpful from the beginning of treatment in interpersonal relationships such as between life partners or in the workplace because the patient becomes much less critical and all at once conflicts and hurts are obviated. The patient is informed that he will be able to talk about everything during the fourth state (verbalization) and that he will prepare for this during the next three stages, with his conflict partner, if this is appropriate. The broadening of what is at first a one-sided perception provides the preparation for the next stage in the process.

Questions in the first phase:

- "By whom are you referred?" to find out about motivation and others involved to treat the client.
- "What brings you to see me?" to understand the symptoms and other reasons for consultation.
- "Can you describe how this influences your life?" to see the impact of symptoms or conflicts.
- "When did this occur to you the first time; when was it worse?" later to compare with the timeline.
- "Can you tell me more about it?" to open a space for free conversation.
- "How could you cope with it?" to find out about activities of the client for help and self-help.

The clients will have the freedom just to talk in an emotionally safe atmosphere and the therapist will at the same time be actively interested and reflecting on the situation and feelings of the client. A balance between spontaneous expression and getting information has to be found. It already shows the possible flexibility of the personality, hope, anxiety, and therapy motivation.

At this time the therapist can write down expressions of the clients on the form or on a free paper with a balance model at the places where it fits before later using the questions of the form to ask the client more systematically in the second phase.

A Patient, Her Situation, and Her Suffering in the First Interview: Case Example [10]

After the sudden death of her father, a high-school teacher, a 24-year-old economics student began to suffer from increasing panic attacks with heart-oriented, vegetative symptoms. Her pre-existing anxiety concerning long travel became stronger until she was completely unable to leave her house alone or, if accompanied, to go any farther away from it than 400 meters. She had not pursued her studies for almost a year and had been required to give up her job as a clerk, which had financed her studies. All medical analyses had produced no evidence of illness. The diagnosis was described as an agoraphobia with panic disturbance (F 40.01 according to ICD 10). The patient came to the therapist at the instigation of her mother and her friend and arrived together with them. This treatment took place in a Southeast European country.

The patient had grown up with her father with whom she could ask about anything. He taught her to trust in exact, rational explanations, in love, and in hope. He was her model, an industrious, conscientious, almost pedantic man, who could pass his understanding of the world on to her through logical principles of order. Her mother's concept was protection and trust above particular friendly, obedient loyalty, and to give motherly tenderness. Through the obedience and loyalty which she showed toward her husband, she remained in the bonding and depended on him for everything. The daughter experienced this harmonic partnership with mutual dependence as a shelter from the outside world and her father's explanations and exactness of dependability as the basis of safety and trust.

First step of the encounter and early use of PPT tools (transcript with thoughts of the therapist):

- *Female patient: "In the autumn it became very bad. I became afraid to go out of the house alone and increasingly also to stay in the house alone."*
- *Therapist (feels empathically, thinks about the balance model, specifically the area of the body): "How did you feel then? Would you describe this for me?"*
- *Patient: "I began to have attacks of pounding heart, very cold hands and feet, I had such trembling, and then my eyes, it was like seeing through fog. And then the pains in my heart. Sometimes they lasted for days."*
- *Therapist (interpreting): "Something terrible was happening with your heart. I understand this as a bodily sign of terrible fear?"*
- *Patient: "Yes, it was as if I was dying every day, but I survived somehow."*
- *Therapist (introducing distancing): "But you can talk about what happened and feel it even though it was so bad."*
- *Patient: "I don't wish such a thing on anybody."*
- *Therapist (wishing to discover the function of the fear, to prepare for a positive interpretation, if possible coming from the patient herself, he already knows the intellectual possibilities of the patient): "What is anxiety for, anyway? Everyone has known anxiety? Why do we have it?"*
- *Patient (considering): "It occurs to me that anxiety protects us from something even worse. If we had no anxiety, what would we actually do? We would have even more wars, I think." And she continues speaking about the*

historical situation of the Southeast European countries and of their newly gained peace.

- *Therapist (thinking that this rationalization is like the patient's intellectual father, wishes to bring the focus back to the patient): "If anxiety is important for an entire people, what would their own anxiety say to them if it could speak?"*
- *Patient: "Until now I have always run from thinking about my father, about his death and what would happen after. With Daddy I felt a strong sense of security. When he was sick, I always avoided any thoughts about him dying (pausing)."*
- *Therapist: "They say, "Anxiety is like a magnifying glass." What comes to your mind about this?"*
- *Patient: "For me anxiety means that my mother can also die one day. Fortunately, she is healthy, but right after my father, my grandfather also died. I was not prepared for this, we had never spoken about anything like this. I can die any time, too. Now I am young and normally don't think about it."*
- *Therapist (thinks that after the patient's own insight into the function of anxiety, her mention of her family and her opening to the theme of the purpose of existence, that she is now more open for the verbalization of attachment and detachment. He thinks of the patient's great attachment to her mother and of the necessity of detachment from the parents and the therapist. He wishes to move to differentiation and thinks of the family resources): "How is your mother in this regard?"*
- *Patient: "I believe … she doesn't show it but I think she is having a hard time with this. She tries to be strong for me."*
- *Therapist: "Could you talk about this with your mother sometime?"*
- *Patient: "Better here, just like the first time when she came. If you are there, maybe she will be able to talk about it. With me she won't."*
- *The therapist thinks that it is a good suggestion to draw her mother into this topic.*

Therapeutic Self-Help Tools

Changing the point of view by positive connotation, transcultural examples, proverbs, and verbal illustration or stories to visualize the experience of the client starts in the first interview to encourage self-help and autonomy as early as possible. Having the patients write down about symptoms like thoughts, body symptoms, and fears, they take a role of the observer for oneself. Providing the patients with a narrative, such as those presented in the book *Oriental Stories in Positive Psychotherapy* [12], which they can share with others, promotes the patient's own activity and feeling of self-esteem. A supportively structured therapeutic process, the unconditional acceptance of the inner suffering, and an attentive understanding of the unconscious desires which lie behind it, all these are the prerequisites for a fruitful therapy. If patient and therapist can develop a sensitivity together through appropriate stories, transcultural examples, and the recognition of the function of anxiety, this is the starting point from which the themes which are hidden behind them can be found.

Stage: Taking Inventory

In this step the information about everyday life of the client, the socio-demographic data, important life events, and the subjective experience are summarized. This is also where the impact which the present problem has had on the patient can be clearly seen, in his hopelessness, etc. The preparations for exploring this can be made either now or in later sessions. Questions about the current life situation in the four areas of the balance model, the distribution of energy, and the significance of specific areas of life on the patient's self-esteem are connected, followed by those relating to the influence his illness has had on the four areas of life, sorted in the balance model (see Fig. 34.1). Many of our colleagues in PPT like to ask in this context about the influence of the illness or problem on the patient and his environ-

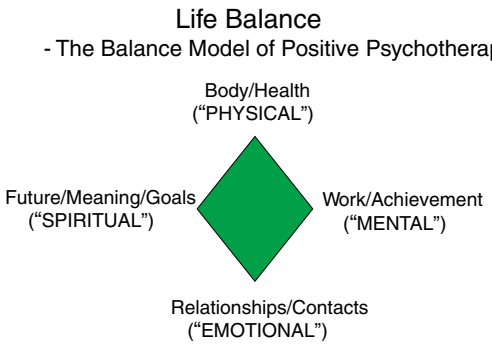


Fig. 34.1 Balance model by N. Peseschkian [1, 17]

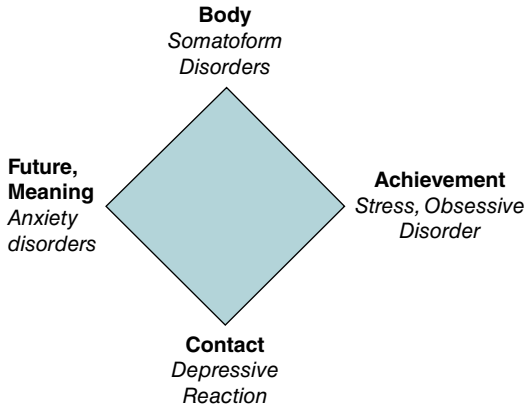


Fig. 34.2 Conflict reactions model (N. Peseschkian) [16]

ment in the four areas of life: What influence has your illness had on your physical well-being, on your work, on your family, and on your view of the future?

This is also where the impact which the present problem has had on the patient can be clearly seen, in his hopelessness, etc. The preparations for exploring this can be made either now or in later sessions. However, what is essential in the first session are the socio-demographic data, the symptoms of the actual complaint, and the questions about stressful life events. Questions about the current life situation in the four areas of the balance model, the distribution of energy, and the significance of specific areas of life on the patient's self-esteem are connected, followed by those relating to the influence his illness has had on the four areas of life (see Fig. 34.2). By means of this graphic presentation, the patient begins to recognize the one-sidedness of his way of life on the one hand and his possibilities for development within the framework of therapy on the other. Visualization is besides structuring an important instrument in PPT for patients to create an own model of understanding of their situation and symptoms. In the first interview, we can ask about the four areas of life:

- Body: Physical activities and perceptions, such as eating, drinking, tenderness, sexuality, sleep, relaxation, sports, appearance, and clothing.
- Achievement: professional achievement and capabilities, such as a trade, household, garden, basic and advanced education, and money management.
- Contacts: relationships and contact styles in partnership and family and with friends, acquaintances, and strangers and social engagement and activities.
- Meaning: meaning and purpose, future and religious practices, meditation, reflection, beliefs, ideas, and development of vision or imagination-fantasy.

With it we can describe:

- The everyday life of the client, how the energy of this client is balanced, and in which areas the symptoms occur.
- The influence of the symptoms on these areas of life, so to say the function of the symptoms.
- Resources and how to encourage them.
- Goals and plans of self-help for the patient.
- Personality capacities (the structure of personality, OPD-2).
- The goals and objectives for therapy.

Patients with psychosomatic disturbances find the balance model which appears in the second stage to be of particular assistance, as the visualization helps to clarify issues and prepare for self-help. These patients are often astonished when they put the events that have happened in

their lives in the past few years into the four areas of the balance model, see them as catalysts, and understand the influence they have had in changing their lives.

Questions to find out the functions of the symptoms:

- How do your complaints affect your physical well-being?
- Can you sleep with your complaints and enjoy food and drink?
- How do your complaints affect your performance?
- Can you work with your pain?
- Can you concentrate on your work when you are restless?
- How does your partner (your family) react when you are in pain?
- How has your love life changed as a result of the pain?
- Who understands your suffering?
- Which people have you been increasingly dependent on since you became ill?
- Which contacts have suffered the most from your illness?
- How do you see your future with your clinical picture?
- Which of your plans have been thwarted by your illness?
- What has changed in your life as a result of the disease?
- What are you most afraid of?
- What do you think needs to change most urgently?[1]

We can put symptoms, conflict reactions, and resources into the balance model during the therapy session. Eventually the patient will distribute them into the four areas and will also thereby attain an understanding of his life situation and of the effects of his disturbance.

The balance model using the four domains of life also presents the basis for the conceptualization of typical conflict models. We flee into illness (somatization) or excessive body-building, into activity and achievement (rationalization in the sense of disturbances of overburdening and adaptation), into refusal to perform, into loneliness or gregariousness (accompanied by idealization or deprecation, which lead to affective disturbances and changes in social behavior), and into fantasy and the world of thought (denial in the sense of anxieties, phobias, panic attacks and disturbances of illusion, addictive behavior, e.g., into lack of imagination [18].

With the balance model, it is possible to understand the therapeutic relation as a mirror of the patients' unconscious contents, when I describe in the four areas, how I feel with the client, to find out what it means (counter transference description).

- Feelings and emotions.
- Rational thoughts.
- Relation patterns with the client.
- Intuition and fantasy.

This way, the balance model helps to classify and understand the therapist's own countertransference.

Differentiation Analytic Inventory (DAI) for Actual Capacities

During this stage, the main task is to recognize the connections, to clarify the pre-history of the individual actual capacities and the preparation for the conflict understanding and resolution, as well as to share the background of the concepts and misunderstandings into order so that she or he can develop a means of understanding them for herself or himself. The positions (attitudes) which as a rule the patient sees as being unchangeable characteristics of the personality will now be viewed as relative according to their meaning in her life history. The significance of the actual capacities is summed up through association by means of an inventory of the actual capacities.

The analysis of the specific content of the conflicts as triggers for the emotions and focuses

[1]Boessmann U, Remmers A, Hübner G: Wirksam behandeln. Bonn 2005 p 75.

in counseling or therapy on the inner and outer conflicts or values lead to the capacities which are the contents of these conflicts. The emotions which lead to suffering or the physical symptoms can then be understood as values functioning in a conflict of opposite concepts. In this connection the conflict-centered process focuses less on the triggers than on identifying and then working through the conflict which caused them.

Application of the Differentiation Analytic Inventory (DAI)

"For whom of you this capacity is more or less important? You can mark it as described. In the last part you can write situations or emotions that come spontaneously in your mind." (see Table 34.1).

Table 34.1 Differentiation Analytic Inventory (DAI)

Actual capacities	I	My partner	Situations, emotions
Punctuality			
Cleanliness			
Orderliness			
Obedience			
Courtesy/politeness			
Honesty/directness			
Fidelity/loyalty			
Justice/fairness			
Diligence/ achievement			
Thriftiness			
Reliability/precision			
Time			
Patience			
Contact			
Love/attachment			
Sexuality/tenderness			
Trust			
Confidence			
Doubt			
Hope			
Faith			

Mark as +++/++/+/+/−

Differentiation Analytic Inventory (DAI) [1]

The Actual Conflict

Capacities represent values, needs, social norms, or virtues that cause feelings and symptoms as a conflict reaction. With the DAI as a scale for social norms (secondary actual capacities, 1–11) and needs (primary actual capacities, 12–21), it is possible to find out about the subconscious associations concerning actual capacities and conscious experiences with microtraumatic events concerning these values [1]. The comparison of patient and partner/children/colleagues brings up more episodes containing these contents. With these the inner, subconscious conflict can be found out comparing actual and earlier episodes.

The Basic Conflict and Role Model Dimensions

One distinguishing feature of PPT is the description and understanding of imprinting situations in early childhood which found basic concepts and can lead to a basic conflict—see Chap. 15 on the conflict model for details. In the First Interview, the patient is asked questions within the context of the four role model dimensions about his relationship to his parents, their relationship with one another, their relationship to society, and their value system. These questions which illuminate the basic situation form the basis on which a primary hypothesis of the possible psychodynamics can be formulated.

Visualization of the early experiences in relation to the emotional important persons widens the classical object-subject perspective to a four-dimensional model:

- "I": The patient's relation with mother, father, or other close relational persons in childhood, specified by asking for the time or patience

these persons took for the client and in which way these close persons had been modelling the client. It represents the object-subject experience of the child.

- "You": The relation of the parents or closest persons with each other offers another perspective on a close person's relation with each other (close object with close object): How has been the influence of the closest partnership around the client? This later subconsciously moderates the own patterns of close relations concerning primary and secondary capacities.
- "We": The observed and experienced relation of parents or closest persons with others like wider family, neighbors, community, or groups (close object with far object).
- "Primal We": As the relation of the parents with the culture, world, spirituality, and life philosophy (close objects with the invisible objects). It was a new construction of Peseschkian; it may be influenced by Frankl, Jung (religion, culture, and meaningfulness of the parents), and Adler (style of the parents).

Personality Style Capacities (Four Areas) and Vulnerable Areas

The visualization of the actual life situation by means of the balance model shows personality traits and states, conflict reactions, or shortcoming areas of life.

The Operationalized Psychodynamic Diagnosis (OPD2) [13] speaks of four observable basis capacities of the personality structure which are similar to the four means of the capacity to know [1].

The questionnaire WIPPF [3] provides a complete summary of the areas of behavior in which the patient and, if appropriate, his partner or conflict partner possess a specific pattern of capacities as by the others positively or negatively evaluated characteristics, and where there are macro- or microtraumas or examples of conflict reaction. The WIPPF also facilitates access to the role models. The most important use of the WIPPF is in individual therapy in which, through the most commonly used evaluation, the patients are brought to an understanding of their resources, conflict contents, conflict reactions, and psychodynamic origins with reference to model dimensions by means of the process of differentiation of contents. The level within oneself, the expectation of the other, and the internalized ideal can be determined for each capacity. From this clarification moments ("Aha!" moments) and episodes in relationships can be associated with them. If the relationship partner, spouse, children, colleagues, or others have also filled out the WIPPF, the comparison clearly shows in which areas there is agreement, and the conflict mostly shows itself to be limited to a small area of actual capacities. A generalization of the conflict, which commonly occurs in partnership, can therefore be avoided. If the partners are actively involved, the WIPPF leads to the acceptance that self-help can be promoted and that therapy for a previous, unconscious conflict can go forward more quickly. The WIPPF can be applied in couple and family therapy as well as in coaching, psychological counseling, and support for children with or without reference persons.

Case Example: Stage 2

The therapist used the balance model in the beginning to present to the patient a complete picture of her life and to recognize her resources and conflict areas. Later her various anxieties are defined as well as the areas in which she felt secure (safe). In the session that is presented here, the patient, while unburdening herself, is able, for the first time, to see her many capacities as resources to be developed.

Later the therapist read her actual capacities from her DAI to her and asked her: "What comes to your mind spontaneously with each of them?"

To cleanliness she answered: "Water, bath"; to obedience, "Careful to a fault"; and to politeness, "to conceal one's own self in front of another person" (not to reveal oneself to another person). She associates faithfulness with "family," justice with "the worthiness of a person," and "trust" with "dumbness." Nothing came to her mind with hope, she replied to time with "immortal," and to both love and sexuality, she said "beauty."

In the following sessions, she illustrated these associations with experiences and lively descriptions of situations. The therapist took the position of one who asks questions, who would like to understand the patient's inner and outer world, while the patient was the one to provide the explanations. Politeness and achievement had been important to Mrs. N. in the present cases; carefulness, lack of trust, misplaced hope, one-sided contact, and sexuality are presumably the areas which will play a role in the therapeutic relationship and in the family, especially in the fourth stage.

After the work of association with the DAI, Ms. N. was introduced to the questionnaire WIPPF. The result was that this patient placed a high value on many primary capacities (except for love and trust), in contrast to the secondary capacities. The role model dimensions showed a very high value for the attachments she had experienced to her father and mother, for a symbiosis between the parents and the strong influence of their philosophy on the patient. During the discussions her protective upbringing and the narrow "togetherness" of the family were made clear. This was the basis for her almost naive-primary expectations and their intrusions into her mind after the death of her father, whose viewpoint toward life had had a strong hold on her through "rationality." The conflict reactions as shown in the WIPPF tended in the direction of "flight to contact and fantasy."

Stage 3: Situational Encouragement

Symptoms, Capacities, and Social Environment as Resources

This stage emphasizes the development of specific resources. The most important aspect of the therapeutic relationship is the reflection of the strengths. This keeps the focus on the available capacities of the patient and his or her reference person. In order to build a new relationship with the conflict partner, the patient and therapist must look at those capacities which speak to this and if possible have the patient name them himself. In any case, the patient and therapist look together at those capacities which correspond to this relationship with the conflict partner and which are important to the patient. In this way, the patient and therapist can work through the meaning of these capacities in the relationship with the conflict partner. Situational encouragement and appreciation are used based on what has been learned during the first and second stages rather than criticism of the partner. The patient and therapist put in order the resources previously established in the balance model as well as areas which have been only briefly touched upon and potentialities and desires which have not yet been fulfilled. The goal is to reach a balance of the energy of life, distributed among the four areas, body, achievement, contact, and meaning-fantasy, as favorable conditions for health and hardiness, and this balance or disbalance can become an instrument of the patient to strengthen areas as resources. The goal is to recover the balance in the four areas.

In this connection, the relationship to the reference person is particularly valuable, whether it is discussed actively or only indirectly, especially in working toward the use of self-help. Information about his specific disturbance and about ways to deal with it, such as the use of medications, relaxation methods, and counseling services, are supportive tasks during this third

stage of the process. N. Peseschkian emphasized that in the third stage, the conflict partner was being prepared to hold up under the criticism which would come his way during the following stage of verbalization. Instead of pointing out a deficit to the patient or those close to him and giving the advice not to aim so high, to begin with, the positive aspects of the one-sidedness are emphasized. The patient is encouraged, and his weakest feeling of self-worth is strengthened in order to create a basis for the analysis of the areas with the deficits. For example, a man who places a high value on achievement and so works long hours every day is not confronted at an early stage with the fact that he should spend more time with his family. At the outset, his inclination for achievement and his motivation to work are seen and identified as a capacity. This is a constructive experience for the patient and is both important and fruitful for building the relationship between the therapist and the patient.

Case Example: Third Stage

Here the focus had been on positive reinterpretation connotation and resources.

Therapist: "I'm glad you're here, how was the trip?"

Patient: "Only my friend is with me today. He's waiting outside in the car. Everything worked out without my mother." (Note: She had been accompanied by her mother until now.)

Therapist: "How does that feel for you?"

Patient: "Somehow better than right after the end of the last session, when I had so much to think about after you told me the story about the man with the grapes and the tiger. I said to myself, in this case of a threatening tiger, while I am in in a dangerous situation, the grapes would never have been sweet for me. I thought how terrible his situation is and wondered how he could be happy after this. But that's how it always is, April showers bring May flowers. I suddenly realized what a dear friend I had! What he has gone through others would not have put up with."

Therapist: (understanding these words as a step forward for the patient and her surprise at her friend's patience as the beginning of differentiation in the direction toward autonomy) "Today you came here entirely alone, without your mother accompanying you. You were not so comfortable with this story before, and I have the impression that you have overcome your reluctance to consider these unpleasant ideas. How does it feel to have done this all by yourself?"

Patient: "Today I really want to take up something from our last meeting again so that I can go through it myself."

Therapist (will set aside his own activities and give the patient an assignment which he has confidence she can do): "If you like, when you are home, you might note down all the things which make you anxious and in another column all the things which in the meantime make you feel secure and safe. When you have written them down, perhaps you can discuss them with your friend. What do you think about this?"

In this way a relationship of trust was constructed during the third stage, which makes open communication possible in the next step, in which also feelings and actual capacities are revisited and early experiences are remembered, organized according to themes and made conscious.

Story Used in Therapy

The Tiger and the Sweet Grapes.

One unfortunate day a wanderer was fleeing from a tiger which was following him. He ran until he reached the edge of a stone wall which he had to climb down. He grabbed onto a thick vine

and hung over the abyss. The tiger growled above him.

Suddenly a terrible growl was heard from below. "Oh, no!" A second terrible tiger was looking up from beneath. The man hung onto the vine—between the two tigers. Two mice, one white and one black, scurried over the calcified rock. They began to chew happily on the root of the vine.

The vine bent low under the burden of the wanderer. But in the sunlight in front of him he discovered a vine covered with small, juicy grapes. Holding on firmly with one hand, he reached out with the other and plucked a fruit and then another and another. He called out: "How tasty these grapes are!" From a Mongolian Tale, adapted by A. Remmers.

Phase 4: Verbalization

In order to move on past the speechlessness, or the outpouring of speech, so common with a conflict, the newly established communication must be carried over step by step into the social environment. In the fourth stage, we discuss both the positive and the negative characteristics of the experiences after which a relationship of trust was constructed during the third stage, which makes open communication possible. This is also the stage in which feelings and actual capacities are revisited and early experiences are remembered, organized according to themes and made conscious. The transference of wishes, expectations, and fears which were experienced with persons earlier in life or feelings which the therapist recognizes as signals indicating painful content are investigated together. This demands that the therapist be open and ready to be a confrontation partner and show a respectful attitude when the patient experiments with changes in behavior within the therapeutic relationship at this early stage. The therapist supports the patient in achieving a balance between politeness and openness and in taking the responsibility for change. The focusing on the themes of the central conflict, the work on the key conflict, politeness-honesty, and the active involvement in the therapy of the reference persons through the patient are the tasks which belong to the fourth stage. The therapist presents the concept of the family group and, if appropriate, brings the family into the therapy. The family concept and unconscious basic conflicts are worked through now. Verbalization means that the time has come for an open discussion after which each recognizes the strengths of the other through observation of the situation, analysis of the contents of the conflict, and mutual encouragement. The conflict partner is now in a position to accept criticism or, at the least, to be able to speak about it. Experience shows that many people are inclined to speak about a problem right away and to hurt the other person by doing so, after which the other person must be strengthened through many hours of encouragement.

Case Example: Fourth Stage—Verbalization

In the *fourth stage*, the patient's unprepared-for bereavement for the father, the main actual conflict, came out. She unconsciously stood between two poles: On the one hand, her father's rational explanations as well as the capacities she had experienced such as dependability, loyalty, and obedience had instilled trust, certainty, and hope in her. On the other hand, she now was experiencing the loss of this clearly ordered, supportive model of her parents' partnership. During therapy this basic conflict presented itself: Emotional warmth and the child's early trust (primary capacities) had become conditional through rational explanations and dependability (secondary capacities) and had not been internalized as

unconditional capacities. Her development of independence generated the concept: "I want to be intellectually independent, therefore, I will ask the one whom I know best, Daddy!" For this patient being independent meant trusting her father blindly. Independent thinking was the same for her as the loss of intellectual support. The patient's unconscious, inner ambivalence and an essential part of the basic conflict which rested upon it was because the fulfillment of primary needs within the family was dependent upon adaptation to secondary capacities.

Thus, an inner conflict was formed: Meaning, support, and trust were all lacking if something was not explained through the intellectual diligence of a dependable partner. The patient developed agoraphobia and panic attacks when alone and in leaving her home as symptoms of her existential anxiety because she had not yet experienced confidence in herself or hope and meaningfulness except in a triangular or couple relationship as being something viable. The compromise, the basic conflict, no longer worked: There is no one like my father to instruct me and no one so responsible to explain things. Because of her symptoms, she was still supported by her mother and her friend, with whom she felt free of symptoms at home. The therapist had been open in this phase and ready to be a confrontation partner and show a respectful attitude when the patient experiments the conflict contents within the therapeutic relationship at this early stage.

Stage 5: Widening of the Goals

The patient is asked to consider the following question:

> What will I do when I no longer have this problem?

This stage accompanies the patient from the first session. It has the function of preventing relapse, leads to a development which is more proactive than reactive, and prevents the patient from going back to using symptoms as means of relief after the completion of a successful therapy. The patient is therefore guided to detach himself from the therapist and to develop new capacities which he had neglected in the past. He will develop micro- and macro-goals together with the therapist. Goals for the foreseeable future can be determined using the balance model.

Case Example: Stage Five—Broadening the Goals

"What will I do when I no longer have this problem?" The seed for this question was planned in the patient's mind from the beginning of the first session on looking into a future, described in four areas of life as a motivation for own active participation in therapy by observing her own feelings and thoughts, writing about stories at home, and talking with relatives and friends about subjects coming up in therapy.

Appendix

First Interview in Positive Psychotherapy.

(by Prof N. Peseschkian, founder of Positive Psychotherapy in co-operation with Hans Deidenbach, Psychologist)

To be completed by the medical doctor/ therapist in collaboration with the patient (his partner, his family).

Last Name: Patient No.: Date of Examination:

Name:

Date of birth:

I. Introductory questions

1.Last name, name, etc.
(to be filled in above)

2. Referral from: .....

3. Attitude of partner (family) towards therapy: .....

4. Future prospects of the patient regarding his/her symptoms: .....

5. Details in short form: .....

Detailed information: questions 70-82

II. Current personal situation of patient (medical information, psychological triggers, reason for his visit)

6. Current symptoms: .....

7. Positive interpretation of symptoms: .....

8. Previous treatment: .....

9. Result of previous treatment: .....

10. Spontaneous comments on symptoms, supposed reasons for symptoms: .....

11. Initial momentum of symptoms:
Sudden occurrence of symptoms/trauma ○
Insidious beginning, due to illness ○
Others: .....

12. Time of first occurrence of symptoms: .....

13. Previous operations, illnesses, hospital stays: ...

14. Risk factors: .....

15. Family anamnesis, illnesses of family members:

16. Physical examination for orientation:
Blood pressure: .....
Weight: .....
Height: .....
State of nutrition:
no abnormality ○ obese ○ very thin ○

General physical condition:
good ○ medium ○ bad ○

General impression:
Corresponds with age ○
Looks younger ○
Aged prematurely ○

III. Psycho-social situation of the patient

Actual conflict (AC)

17. What has happened during the last 5-10 years?
Please name 10 events (incidents).....
.....
.....
.....

18. Starting point of actual conflict (year):
AC before beginning of symptoms ○
AC before increase of symptoms ○
AC after symptoms ○

Influence of illness and difficulties on the patient and his surroundings

19. Which impact the illness has had on you general well-being?
strong ○ moderate ○ no impact ○
Spontaneous answer: .....
.....
.....
.....

20. Which impact the illness has had on your job?
Strong ○ moderate ○ little ○
no impact ○ not relevant ○
Spontaneous answer: .....
.....
.....

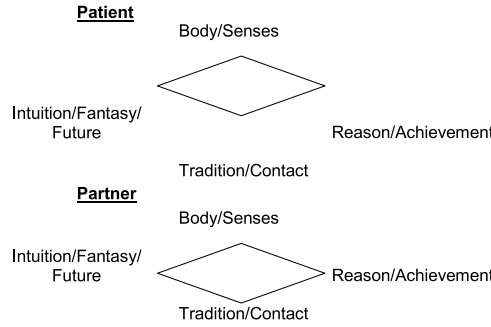
21. Which impact the illness has had on your partner and your family?
strong ○ moderate ○ little ○
no impact ○ not relevant ○
Spontaneous answer: .....
.....
.....
.....

22. Which impact the illness has had on your relationships and contacts?
strong ○ moderate ○ little ○
no impact ○ not relevant ○
Spontaneous answer: .....
.....
.....

23. Which impact the illness has had on your attitude towards your own future and future prospects?
strong ○ moderate ○ little ○
no impact ○ not relevant ○
Spontaneous answer: .....
.....
.....
.....

24. Did you recently have the sense of inner unrest or inner tension?
yes ○ no ○ I don't know ○
Spontaneous answer: .....
.....
.....
.....

The four areas of conflict management (capacity to know)



Area: Body/Senses

25. What is the importance of health and body issues for you? For your partner?

- very important (vi), important (i), moderate (m), not important (ni)

	+2 sw	+1 w	-1 m	-2 uw
Body care / Aesthetic:	○	○	○	○
Sports:	○	○	○	○
Relaxation:	○	○	○	○
Nourishment:	○	○	○	○
Sleep:	○	○	○	○
Sexuality:	○	○	○	○
Body contact / Tenderness:	○	○	○	○
Pain:	○	○	○	○

Area: Achievement

26. What is the role and meaning of your profession and job for you? For your partner?
very important ○ important ○ moderate ○
not important ○
Spontaneous answer: .....
.....

27. Are you happy with your present job situation? Your partner?
yes ○ fairly happy ○ no ○
Spontaneous answer: .....
.....
.....

Area: Contact

28. What is the role and meaning of social contacts for you (guests, friends, family, neighbours, colleagues and fellow men)? For your partner?
very important ○ important ○ fairly important ○ not important ○
Spontaneous answer:.....
.....
.....

29. Do you pay attention to social events? (Politics, clubs, environmental protection, citizens' initiatives) Your partner?
very important ○ important ○ fairly important ○ not important ○
Spontaneous answer:.....
.....
.....

30. Do you have contact with people from different cultures and backgrounds? Your partner?
often ○ sometimes ○ seldom ○ never ○
Spontaneous answer: .....
.....
.....

31. Which aspects attracted you by people from a different cultural background? Your partner?
Spontaneous answer: .....
.....
.....
.....

32. How do you see the international problems as well as possibilities in the encounter with people of diverse culture and background?
Spontaneous answer: .....
.....
.....
.....

33. Are you satisfied and happy with your partner relationship? Your partner?
a lot ○ averaged ○ insufficient ○ no ○
Spontaneous answer: .....
.....
.....

Area: fantasy/future

34. Do you think in general about your own future and the future of your family? Your partner?
a lot ○ sometimes ○ rarely ○ never ○
Spontaneous answer: .....
.....
.....

35. Are you worried about your future professional career? Your partner?
yes ○ no ○ not relevant ○

36. Do you also think in general about the distant future and about the global future of mankind? (World crisis, war, world peace) Your partner?
often ○ sometimes ○ rarely ○ never ○
Spontaneous answer: .....
.....
.....

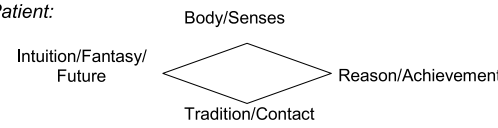
37. Are you hopeful towards future? Your partner?
yes ○ no ○ I don't know ○
Spontaneous answer: .....
.....
.....

38. Do you deal with questions concerning the meaning of life, of death or of life after death? Your partner?
often ○ sometimes ○ rarely ○ never ○
Spontaneous answer: .....
.....
.....

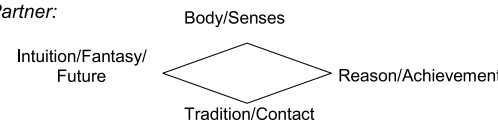
39. Do you believe in life after death? Your partner?
yes ○ no ○ I don't know ○
Reason: .....
.....
.....

40. Result of "4 areas of dealing with conflicts":

Patient:

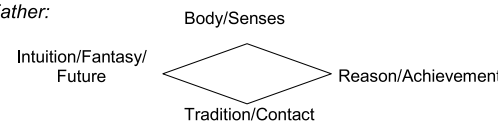


Partner:

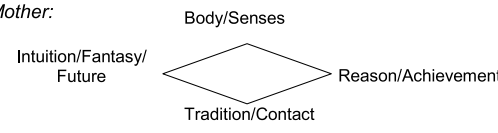


Parents:

Father:



Mother:



Basic conflict:
(The 4 model-dimensions of capacity to love)

41. People who were responsible for the upbringing of the patient:
both parents ○ single parent ○
children's home ○ others ○
Spontaneous answer: .....
.....
.....

42. Were your parents married when you were born?
yes ○ no ○
Spontaneous answer: .....
.....
.....

43. To whom did you have a special or closer relation in your childhood?
mother ○ father ○ both parents ○
family members ○ others ○ nobody/none ○
Spontaneous answer: .....
.....
.....

44. Who spent more time with you in your childhood?
father ○ mother ○ others ○
Spontaneous answer: .....
.....
.....

45. Who was more patient with you?
father ○ mother ○ others ○
Spontaneous answer: .....
.....
.....

46. Who was your example?
father ○ mother ○ others ○ nobody ○
Spontaneous answer: .....
.....
.....

47. Relationship to siblings
good ○ ordinary ○ mixed ○ bad ○
no relationship ○ not relevant ○
Spontaneous answer: .....
.....
.....

48. How did you feel about your parent's marriage in your childhood?
very harmonious ○ harmonious ○ ordinary ○
bad ○ not relevant ○
Spontaneous answer: .....
.....
.....

49. Which importance did your parents attribute to the areas work and achievement?
very important ○ important ○
not very important ○ not important ○
Spontaneous answer: .....
.....
.....

50. Did your parents enjoy contact with other people?
yes (very much) ○ not very much ○
a little ○ not at all ○
Spontaneous answer: .....
.....
.....
.....
.....

51. Did you talk about issues such as "meaning of life", "death" or similar topics at home?
often ○ sometimes ○ rarely ○ never ○
Spontaneous answer: .....
.....
.....
.....

52. What was considered to be the meaning of life in your family? (Health, work, family / fellowmen, religion)
Spontaneous answer: .....
.....
.....
.....
.....

53. Which motto did you have at home?
Spontaneous answer: .....
.....
.....
.....
.....

54. Which proverbs, metaphors and concepts are most important to you?
Spontaneous answer: .....
.....
.....
.....
.....

55. Who is your favourite author?
Spontaneous answer: .....
.....
.....
.....

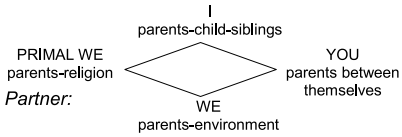
56. Who read or told you stories? (Father, mother, grandparents, nursery-school teacher, etc.)
Spontaneous answer: .....
.....
.....
.....

57. Can you remember situations in which you were told stories: How did you feel about that?
Spontaneous answer: .....
.....
.....
.....
.....

The findings of the 4 areas of model-dimensions / capacity to love (basic conflict)

58. Explaining the diagnosis to the patient and his family:

Patient:

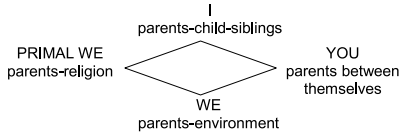


Partner:

Relationship of parents and siblings to patient

	Mother	Father
Time		
Patience		
Model		

Partner:



Relationship of parents and siblings to partner

	Mother	Father
Time		
Patience		
Model		

59. Differentiation Analytical Inventory (DAI) – Short Form

Actual Capabilities	**Patient** + -	**Partner** + -	**Spontaneous Answer**
Punctuality			
Cleanliness			
Orderliness			
Obedience			
Politeness			
Honesty Sincerity			
Fidelity			
Justice			
Diligence Achievement			
Thrift Economy			
Reliability Exactness			
Love			
Patience			
Time			
Trust Hope			
Contact			
Sexuality Sex			
Belief Faith			

The findings of "Differential Analysis Inventory"

60. Description of DAI for the patient and the patient-family in regards of "inner conflict".....
.....
.....
.....
.....
.....
.....

61. Which part of the first interview was important to you? Would you please explain that in your own words?.....
.....
.....
.....
.....
.....
.....
.....

62. Explanation of the further procedure. (Plan of therapy taking the 5 stages into consideration): .....
.....
.....
.....
.....
.....
.....

63. Are you a good listener?
Spontaneous answer:.....
.....

64. Are you able to ask precise questions?
Spontaneous answer:.....
.....

65. Are you able to encourage yourself and other people?
Spontaneous answer:.....
.....

66. Are you able to explain your problem to others in a suitable manner?
Spontaneous answer:.....
.....

67. Are you able to appear optimistic despite your problems?
Spontaneous answer:.....
.....

68. What are your desires and wishes for the next 5 years?
Spontaneous answer:.....
.....
.....
.....

69. Diagnoses, medication: .....
.....
.....
.....
.....
.....

IV Socio-demographic data

70. Age: ..... years
Partner: Age: ..... years

71. Sex: m. ○ f. ○

72. Marital status
married ○ since: .....
single without stable partner ○
single with stable partner ○
divorced ○
widowed ○
separated ○
remarried (after death of partner, divorce) ○
widowed or divorced but living together with stable partner ○

73. Number of children: .....
Age of children:.....

74. Profession (education):.....
.....
.....
.....

75. School-leaving qualifications:
Special school for mentally / physically disabled ○
primary school ○ secondary school ○
a levels ○ graduate / university ○

76. Current job and achievement:.....
.....
.....

77. The partner's job: .....
.....

78. Parental family situation of the patient:
Both parents alive ○
When did they marry?: .....
Age of father:.....
Age of mother:.....
Only one parent alive ○
Who?:.....
Year of death of late parent: .....
Both parents have died ○
Year of death: Mother:..... Father: .....
Other family situation: (i.e. foster parents, adoptive parents, children's home) ○

79. Parents' professions:
Father: .....
Mother: .....

80. Number of siblings: .....
Age of siblings: .....

81. Religion:
which?.....
No religious affiliation: ○
Has left the Church ○

82. Nationality/Ethnicity: .....

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The Conflict Model of Positive Psychotherapy

35

Maksim Goncharov

Introduction

The general meaning of conflict (from the Latin: *confligere*, to clash, to strike together) refers to the concurrence of different positions within one person (an inner struggle between motives, wishes, values, and ideas) or between several persons (Brockhaus German Encyclopedia, 1990) [1, 2]. Conflicts are universal phenomena. However, the conflict is not a quarrel or a scandal, as is often understood by people. Rather, it is the difference between the desired and what is actually happening. Conflicts happen all the time. In other words, conflicts happen constantly, they are always with us. However, not all of them have a significant impact on us. Therefore, conflicts need to be differentiated into useful categories. In Positive Psychotherapy (PPT), there are four kinds of conflict: actual conflict, basic conflict, internal conflict (inner conflict), and a key conflict. The conflict model of PPT has strong internal consistency and practical application, thus becoming a useful clinical tool to help clients to work through problems.

Key Definitions

Actual Conflict

In his books, psychiatrist Nossrat Peseschkian describes the actual conflict as acute or chronic situations happening at the present time; for example, professional changes or circumstances, family events such as divorce or marriage, financial difficulties, deaths of close people, etc. [3–7]. However, an actual conflict is not just an event, but also refers to situations that have been caused by this event (Fig. 35.1).

The problematic situation could have begun a long time ago, for example, some years or even decades, but retains its actuality until now. For instance, entering family life, divorce, changing workplace, or moving somewhere can trigger the situation of chronic dissatisfaction and impact the quality of life. The event that starts an actual conflict initially is neutral and gets its personal color in accordance with its importance for the person. This importance is determined by the touched values (actual capacities) that were transmitted to us in the process of rearing, when we were children [3, 8, 9].

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Fig. 35.1 Actual conflict

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_35

Not every situation that does not satisfy our expectations creates noticeable conflict. For example, to be late for a bus and the loss of a close friend are not the same things. As any other conflict, an actual conflict is based on a contradiction of values. The conflictual stress takes place if the current life situation affects important values and needs that were developed in childhood [10–12].

The more significant and deeper the value connected with the person's perception of her/himself is, the more noticeable inner demands (expectations) it creates; as a result, its nonsatisfaction creates more salient disappointments [3, 13, 14]. For example, a person who lives with the principle "I am punctuality itself" experiences greater conflictual stress when meeting the unpunctuality of another person or his own. Thus, actual conflict is always an external, current situation. The importance of the actual conflict depends on the degree of discrepancy between the expected and what is actually happening, or better to say, observed. The greater the discrepancy, the more the conflict becomes actual, the greater conflictual stress it creates, and the more tangibly it is experienced by the person [15–17]. Graphically, it can be expressed as shown in Fig. 35.2.

Thus, we arrive at the fact that the ***actual conflict is the current conflictual (problematic) life situation, which leads to emotional stress caused by the mismatch between the expected and the observed.*** An actual conflict is also an external situation that comes and checks on the intensity of our internal values [16].

The actual conflict itself is not something pathological. It can exist independently of the current problematic situation and may also become a trigger for the activation of the basic conflict with the formation of further conflict dynamics. Therefore, divorce, for example, can reactivate for women the early loss of her father, because she loses her lover again. Here, the actual conflict reopens old wounds and tests the existing family concept; thus, it gives extra energy to this conflict [5]. But we will discuss this more later.

At the same time, there may be several actual conflicts, and they can be ranked in importance; for example, a problem at work, health situation, difficulties in the family or in relationships. Normally, we cope quite well with our actual conflicts.

An actual conflict is often a consciously understood conflict. This means it can be named and described in words. In this regard, it may be processed and resolved. However, the actual conflict may be unconscious, for instance, in psychosomatic disorders, anxiety and psychotic disorders, in addition to dependencies and some other situations. In these cases, the person is only aware of the symptoms; he/she is fixated on the sufferings and can describe them in detail, but does not understand what causes these symptoms. For the client, the connection of symptoms with the conflictual life situation may be missing.

In clinical practice, unawareness of the Actual conflict may also serve as a diagnostic criterion and indicate lower psychological mindedness, and either the impact of psychological defenses or reduced structural integrity [16, 18, 19].

The conflict always creates energy. If this energy for whatever reason does not have an output, it forms conflictual tension, which can lead to the formation of symptoms and disorders (Fig. 35.3).

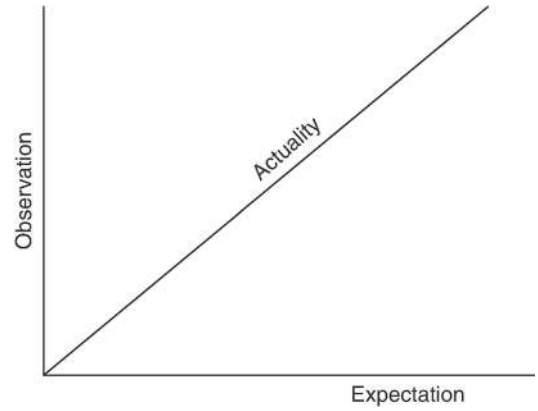


Fig. 35.2 Conflict actuality

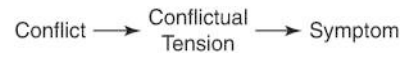


Fig. 35.3 Conflictual tension

In psychotherapy procedures, we try to go backwards and take a step back from symptoms of the conflict and thus get an opportunity to work through the cause of the symptoms.

Not every conflictual situation happening in a person's life generates this conflictual tension. Whether it arises or not depends on the readiness of a person, which is determined by his/her basic conflicts. In other words, the energy of an actual conflict is not enough to generate the conflict dynamics and shape the symptoms of the disorder. It needs an additional energy, which is derived from the basic conflict.

The Causes of Actual Conflict

How did the actual conflict arise? Peseschkian describes two major categories of causes of actual conflict: macro-trauma and micro-trauma (Fig. 35.4). In the first case, the link between the causes and effects is relatively easy to trace; in the second, it may be very difficult.

Macro-traumas are life events, such as loss of job, bankruptcy, relocation, death of loved ones, etc. These are fairly large events, which for a while will derail almost any person. It is important to bear in mind that it is not just about the negative but also positive events as well [20, 21]. Typically, a person can relatively easily describe the impact of this event on his life situation and tell us the exact time of its occurrence. When psychiatrists question the patient about life before the disorder, they are in fact, trying to find these macro-traumas. If any are found, it is likely to be classified as a psychogenic or exogenous disorder. The magnitude of stress and the extent of the negative impact depend on the individual and cultural features of a concrete person [17, 22–24]. This should be considered in clinical application with clients.

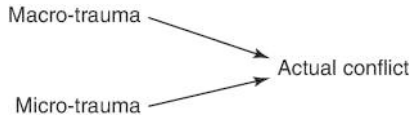


Fig. 35.4 The causes of actual conflict

The second category is micro-trauma. In contrast to the first category, this is an accumulation of insignificant events, for example, unpunctuality of a partner, unreliability and injustice of subordinates, disobedience of a child, etc. As you probably noticed, these micro-events can be described in terms of actual capacities. It is such a trivial event that life operates on the principle of "constant dropping wears away a stone." Opposite to macro-trauma, a person cannot accurately describe the time of this event's appearance, and argues rather for broader temporal categories: "we had been fighting over the past few years", "our relationship changed about 5–6 years ago", etc. Recurring episodes of conflicts lead to re-traumatization, deplete resources, actualize the situation, and make it chronic. An actual conflict can arise as a combination of external stress, overload, and personally determined capacities [4, 25].

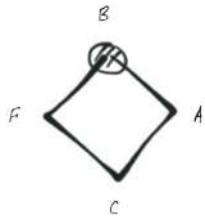
Operationalization of Actual Conflict

As PPT is a psychodynamic method, we do not dwell just on the symptoms, such as fear or depression, but try to answer the question: "What is behind these symptoms that constitute the content of the conflict"? Like any psychodynamic construct, an actual conflict needs diagnosis and operationalization. The operationalization of the actual conflict is based on three aspects of the diagnosis:

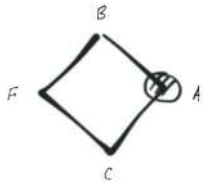
1. Localization
2. Content
3. Reaction to conflict

Localization

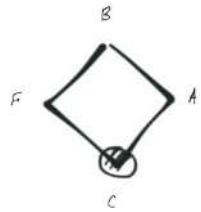
Localization of the actual conflict is a domain where the conflict predominantly takes place. To describe the localization of the actual conflict, we use the balance model of PPT with the four areas/dimensions/spheres of life (body/health, activity/achievements, contacts/relationships, and meaning/future).



Picture 35.1 The Actual conflict is located in the domain of body/health. (*A* achievements, *B* body, *C* contacts, *F* future)



Picture 35.2 The actual conflict is located in the domain of activity/achievements. (*A* achievements, *B* body, *C* contacts, *F* future)



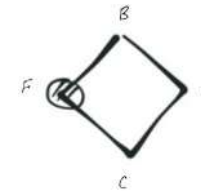
Picture 35.3 The actual conflict is located in the domain of contacts/relationship. (*A* achievements, *B* body, *C* contacts, *F* future)

Actual conflict in the sphere of body/health: trauma or disease, informing of the diagnosis, dysfunction of an organ or system, dissatisfaction of one's own appearance, etc. (Picture 35.1).

The actual conflict in the sphere of activity/achievements: problems in the workplace (being judged as not good enough, not being promoted, not being paid well), the problems regarding subordination or loyalty, etc. (Picture 35.2).

The actual conflict in the sphere of contacts/relationship: tense relationship, misunderstanding with the partner or parents or other close ones; quarrels in the family, loneliness, inability to establish a dialog, etc. (Picture 35.3).

The actual conflict in the sphere of meaning/future: loss of orientation, death, meaning or goals, anxiety, deadlock, collapse of ideals, etc. (Picture 35.4).



Picture 35.4 The actual conflict is located in the domain of meaning/future. (*A* achievements, *B* body, *C* contacts, *F* future)

Thus, the localization of the actual conflict helps us to identify and localize the area of life in which there is a discrepancy between the expected and the observed [16].

Content

The actual conflict always revolves around some values [26–28]. These values in PPT are described in terms of *actual capacities*. In its psychological content, these capacities are divided into two categories: the secondary and primary capacities.

Secondary capacities are associated with the transmission of knowledge and, thus represent a basic capacity to know. They reflect the values and norms of the social group of the individual.

Primary capacities represent the basic capacity to love, and they form from the first day of a person's life owing to contact with other people. See Table 35.1 for a list of primary and secondary capacities.

Besides the above-mentioned functions, the actual capacities represent the core conflict theme as well. It means that a client can be traumatized by these social norms or values in their current life situation. When the expected behavior of someone does not correspond with behavior observed, it makes the client experience a conflictual tension [29–31].

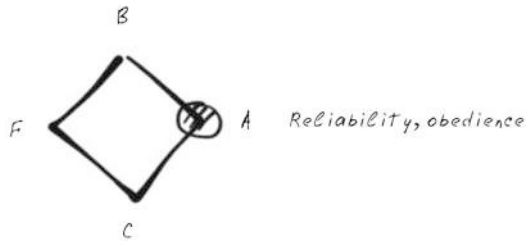
Example: the actual conflict is located in the domain of achievements, in the content of actual conflict: reliability and obedience (Picture 35.5).

This means that the client, in the domain of achievements, for example, expects from someone greater reliability and loyalty.

As a rule, the content of the actual conflict may be described by the secondary actual capaci-

Table 35.1 Actual capacities

Primary capacities	Secondary capacities
Time	Punctuality
Patience	Cleanliness
Contact	Orderliness
Love/acceptance	Obedience
Sexuality	Courtesy
Trust	Honesty
Confidence	Accuracy
Doubt	Faithfulness
Hope	Justice
Faith	Diligence
Model	Thrift
Unity/integrity	Conscientiousness



Picture 35.5 Localization and the content of actual conflict. (*A* achievements, *B* body, *C* contacts, *F* future)

ties, as they are behavioral norms and may be relatively easily observed. Nevertheless, the primary actual capacities can be in the content of actual conflict as well. For example, patience, sexuality, trust [3, 5, 6, 32].

The content of the actual conflict is not always possible to determine immediately from the client description. For example: "You do not respect me." The claimed actual capacity: acceptance/love. But if the conflict partner does not openly express his/her disrespect, it means that the client interprets their partners' behavior as disrespect. In this case it requires clarification. For example: "You do not respect me, because you are late and make me wait" (punctuality). Or "You do not respect me because you do not obey me." In this case, "obedience" serves as a criterion of respect.

It is important to emphasize that actual conflict always has content that is the theme or value, which, in this life situation does not get the expected satisfaction and thus is frustrated. Further analysis clarifies the reasons for such vulnerability of those actual capacities involved in the conflict. The ability to see and name the actual conflicts can serve as good diagnostic criteria for good structural integration and psychological mindedness [2, 16, 33].

Reaction to Conflict

Despite all the cultural and social differences and peculiarities of each person, we can observe that all people apply the same forms of typical conflict responses to overcome their problems [34, 35]. When we have problems (we get angry, become depressed and feel misunderstood, live in constant tension or lose the meaning of life), all these difficulties can be expressed in these four spheres of life. It gives a picture of how people perceive themselves and the world around and how recognition and the reality control happening. In other words, these four aspects of life, respectively, are used to process our conflicts [7, 36]. Thus, we come to four ways of dealing with the conflicts (Fig. 35.5):

1. *"Escape into disease" or "escape into body/symptoms"* implies that the client in response to the conflict begins to experience ailments, begins to care more about the body and health, tries hard to lose weight, exercise, or trim oneself down. Physical reactions to conflicts can be the following: change of physical activity (sports—"to hold on, stop getting old"), sleep ("sleep over" the conflicts, sleep disorders), food (gluttony—"eat over the grief", refusal of food—"slimming mania"), sexuality (Don Juanism, nymphomania or abstaining from sex), functional disorders, and psychosomatic reactions. In addition, the episodes of secondary gain from illness could be classified as "escape into illness."
2. *"Escape into activity"* involves a situation in which, in response to the conflict, a person increases his/her activity and productivity, seeks to achieve greater results, loads work upon himself, tidies the apartment, takes on increased obligations at work, etc.

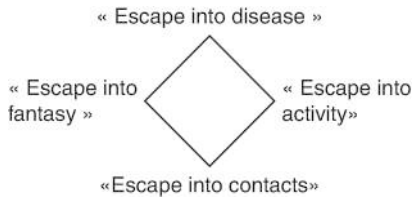


Fig. 35.5 Four ways of dealing with the conflicts within the balance model of PPT

3. "*Escape into contacts*" is manifested in the search for companionship to resolve the conflict: meeting friends, chatting on the phone with parents, the use of social networks, etc.
4. "*Escape into fantasy*" means living in dreams or another less painful and more attractive reality. It can be reading books, watching movies, or playing computer games. In addition, this includes the use of alcohol or drugs, and suicidal behavior as well.

Normally, all the ways are available to us as a resource of conflict processing. For example, in response to a problematic situation we can become sad or annoyed (body/senses). Thus, we are able to recognize and express our feelings. We can do something about it; we can do something useful (activities/achievements). We can also share our experiences with others, escape, seek for support and assistance (contact/communication). We can think about the consequences, invent a new plan or a new meaning (future/fantasy). That is, having at our disposal the ability to use all four domains of life as a resource, we can go through the difficulties much easier. Adequate processing of actual conflicts allows us to resolve problem situations on time and to maintain the balance and good quality of life. For example, if your health has deteriorated, it makes sense to give it some more attention, get the necessary examination and treatment. Even if the event is experienced as something joyless, targeted investment of energy to the conflict area allows us to get out of it with minimal harmful effects.

Despite the seeming naturalness of the above-mentioned approach to solving the difficulties, we do not always act rationally [16, 37, 38]. For example, having a problematic situation in a partnership, we can load ourselves with work, trying to minimize communication with the partner. It means that the actual conflict unfolds in the domain of contacts, but is processed by "escaping into activity." Or, on the contrary, the client has serious problems at work, and instead of solving them at work, he complains about them to a friend or partner ("escape into contacts") or tries to be distracted by using alcohol ("escape into fantasy"). This type of conflict reaction is called "shifting". The reasons for such reactions should be sought in the family concepts.

Even if a conflict arises in the relationship with one partner, and is processed in a relationship with another partner, this cannot be considered as an adequate investment in conflict resolution. Despite the fact that we can get some relief and support from such a way of conflict processing, sooner or later we will have to put our efforts into the domain and into the content that formed the actual conflict. This is a guarantee of effective conflict management. It means that the actual conflict should be resolved in the domain where it originated, with the object with which it originated, and on the subject around which it originated. Because in fact we cannot avoid all conflicts, we should strive to learn how to manage them.

An actual conflict encourages people to seek the way out, and they make it with the means available using the four domains of life. In clinical application, four ways of processing the conflict help us to identify how exactly our client is doing this [3, 36]. Here we estimate how the client responds to the unfolding actual conflict using four ways of processing the conflict, and what areas of life are mainly involved in the processing of the actual conflict. For example, in response to the conflict in the domain of achievements about reliability and obedience, the client tends "to escape" into chatting and discussing the conflict with loved ones.

To be able to accurately diagnose the actual conflict, it must be formulated in the shape of a short description. For example: "When I repeatedly explain to my subordinate what I expect from him, but he is unable to fulfill it, I get mad and won't calm down until I talk to someone about it."

With regard to the formulation of an actual conflict, it is located in the domain of achieve-

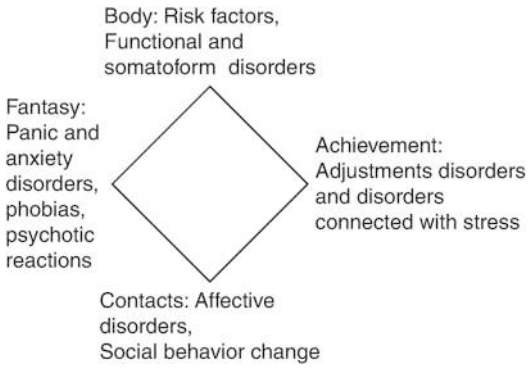


Fig. 35.6 Spreading the mental and psychosomatic disorders in the model of four spheres of life

ments (conflict with a subordinate). It is processed mainly by escaping into communication (contacts) and emotional reactions (body). The content of the actual conflict consists of conscientiousness and obedience (high expectations versus low performance).

Because the client is able to share his/her emotional experiences with others, it can reduce the conflictual tension and return to normality.

The conflict and reaction to conflict should be clearly differentiated. The problem is that the conflict can be in one area, such as "contacts," and can be processed in a completely different area, such as "fantasy," for example. In this case, the conflict has very little chance of resolution, because it remains intact.

Headaches, gastrointestinal disorders, sleep disorders, rheumatic diseases, pain, asthma, cardiac pathology, sexual disorders, addiction, phobias, depression, obsessive–compulsive disorder, etc., are today considered in terms of experiencing and processing the mental and psychosocial conflicts. In this regard, all categories of mental and psychosomatic disorders can be distributed in this model (Fig. 35.6).

Key Conflict

In the work of Nossrat Peseschkian, the key conflict is described quite simply. However, it plays a significant role in the regulation of conflict reactions and even in symptom formation (Fig. 35.7).

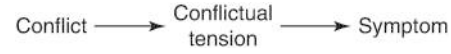


Fig. 35.7 Conflictual tension

Every conflict always creates energy. If for some reason this energy has no outlet, conflict tension is formed, which, under conditions of further blocking, can lead to symptoms and disorders.

When conflicting tension is already formed, it should be somehow utilized in the near future. Essentially, there are only two alternatives: to go out into the external world or to stay inside. Here is the so-called "switch-on point," where the further direction of conflict processing needs to be determined [7] and a new field of conflict will be formed. After the actual conflict receives its individual meaning, the experiencing of this conflict and the response to it are facing another conflict, which has been named a key conflict. Thus, the key conflict exists, only when there is an actual conflict.

A key conflict is called key, because it is like a key that opens or closes our reaction to the conflict (Fig. 35.8). There is a choice between revealing the reaction to the conflict to others through "honesty" and hiding it away from them via "courtesy."

Substantively, the KC is a dichotomy of two capacities: honesty (openness, candor) and courtesy (politeness). In a sense, the key conflict is like a switch that directs our reaction to outside or inside. Getting into a conflict, we have a choice: to respond openly (outward) or react hiddenly (inside).

On this basis, "honesty" or "directness" is understood as an ability to openly express one's own needs, true thoughts and feelings, regardless of the consequences and reactions of others, an ability to say "No" and the ability to reject, stand up for oneself and to assert, allowing the risk of aggression. Example: "I always say what I think, regardless of whether they like it or not." (Fig. 35.9).

Besides all this, "honesty" is one of the conditions of verbalization. It makes an individual's experiences available for evaluation. However, if

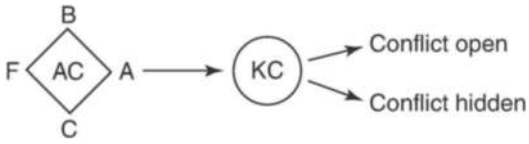


Fig. 35.8 Actual and the key conflicts

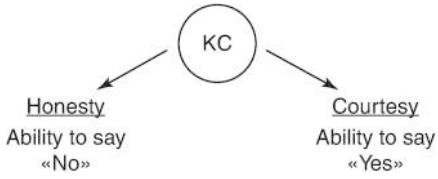


Fig. 35.9 The potential outcomes of the key conflict

honesty/directness is too much, it can also prevent proper communication. In addition, honesty is an ability to stay in contact with yourself, to be honest regarding your own true manifestations (revelations). Hence, sincerity is an integral part of congruence and authenticity.

The second component, "politeness" or "courtesy", is understood to be an ability to suppress aggression toward the social environment, the ability to avoid confrontations, the ability to consider the attitude of the other, the ability to say "Yes" or the ability to accept with the cost of intuitive failure and potential emotional reactions of fear. Example: "I'm afraid to openly express my opinion, because I don't want to lose my good image in the eyes of others." "Politeness" can be understood as the need to stay in contact with others, the ability to maintain contact.

These two abilities, honesty and courtesy, are responsible for whether the conflict will be solved or whether it will be further developed and enhanced.

As the conflict is always a clash of interests between the two parties, for effective conflict management it needs participation of the second party: any individual, or other needs of the same person. For this, the conflict content should be comprehended and verbalized; this gives a chance for adequate perception of the conflict content of the other party.

From here we can distinguish three possible perspectives or patterns of conflict processing (Fig. 35.10).

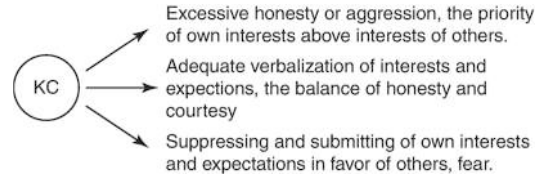


Fig. 35.10 Three possible perspectives of key conflict processing

Through the use of courtesy in the key conflict, a person consciously suppresses aggression and thereby creates an internal conflictual tension, which may also eventually lead to symptoms and disorders.

Most psychotherapy patients process their conflicts through the use of courtesy. In most cases, this is because politeness often lets one avoid escalation or minimize situational conflicts.

If "politeness "were to be compared to the walls of a vessel into which minor conflicts (micro-trauma) fall, and "honesty" to a threshold of a vessel or a hole through which the content can be expelled, it can be seen that the higher the threshold, the more intense the manifestation of honesty will look like. With a significant accumulation of honesty, it may look like a large wave or even a tsunami: intensive, aggressive, and sometimes destructive. In this case, the "honesty" will be forced and will be accompanied by a variety of complicated feelings (Fig. 35.11).

If the threshold of honesty is to be reduced, its intensity can be significantly lowered, and it will be easier to manage (Fig. 35.12).

From the examples that Nossrat Peseschkian offers in his books, it follows that the key conflict is conscious, because clients are able to verbalize the reason why they are either open (honest) or closed (polite).

The KC processes conscious content; however, it, in fact, is the inner conflict and can also lead to symptom formation because it can produce such strong emotions as fear or anger. However, not every IC is a KC. In contrast to the IC, as we have seen, the KC is its conscious analog. To open or to close something, you must be aware of what you are going to open or close (Fig. 35.13).

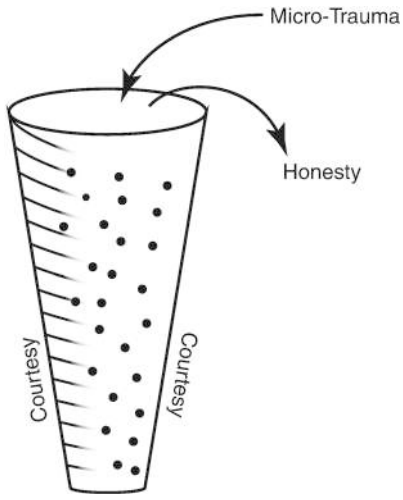


Fig. 35.11 High threshold of honesty

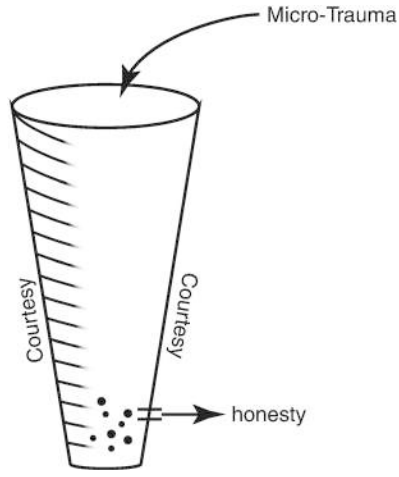


Fig. 35.12 Low threshold of honesty

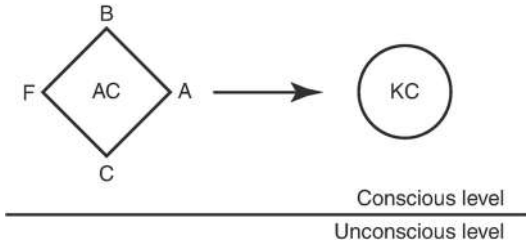


Fig. 35.13 Actual and the key conflicts and levels of awareness

Thus, ***the key conflict is a conscious inner conflict between the need to express one's own interests (openness/honesty), and thus remain in contact with yourself (congruence) and the need not to compromise the attitude to yourself and thereby to stay in contact with the other (courtesy/politeness)*** [16]**.**

In this regard, the key conflict can be called a conflict of conflicts, as it always occurs when there is a conscious choice creating tension: to announce the significant conflict of interests or hide it. Equal accessibility of these two capacities to deal with conflict allows one to maintain the balance of contact with yourself and with others.

In the model of PPT, we consider the key point "courtesy (politeness)–honesty (sincerity)" the most vulnerable place for the following scheme of symptom occurrence: the reactions of courtesy correspond to the endocrine and mediator mechanisms of the central nervous system (CNS) and the reaction of fear; the reactions of honesty in the CNS correspond to aggression [6, 7]. Honesty taken to an extreme extent may look like aggression. At the same time, aggression is substantially connected with honesty. Physiologically, honesty activates the sympathetic nervous system (SNS) and requires from the person an active position and readiness for action. Accordingly, the potential problems of this person are most likely to occur in the area of contacts and achievements (Fig. 35.14).

Politeness to an extreme extent looks like fear. It activates the parasympathetic nervous system (PNS) and leads to a passive–defensive position. Accordingly, the potential problems are most likely to be located in areas of the body and fantasies, i.e., psychosomatic and phobic or anxiety disorders.

Thus, if sincerity sharpens the conflict, politeness makes it chronic. However, even toughness and too straightforwardly telling the truth provide much more of a chance to understand the complaint and respond to it adequately than excessive politeness.

The key conflict processes the conscious content, i.e., the current conflictual life situation (actual conflict) or conscious family concept (basic conflict) may be verbalized (announced).

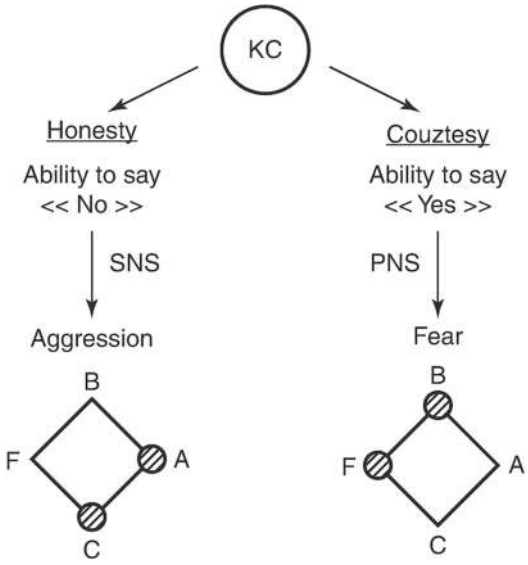


Fig. 35.14 The key conflict and symptom formation

To assess the patient's conflict position and his/her communication capacities, we look for examples of their experience and attitude toward openness–courtesy described in specific situations [39, 40]. A key conflict, in the process of psychotherapy, is actively discussed at the stage of verbalization, when a greater understanding of conflicts and their contents has been achieved. In this regard, the emergence of a key conflict can be viewed as an opportunity to articulate one's own feelings; whereas keeping contact with others is a positive sign, verbalizing this conflict is a sign of increased awareness of the client. The awareness gives much greater choice and expands the repertoire of responses.

Basic Conflict (BC)

The causes of our failures are often our so-called "life concepts," which are indeed our inherited family concepts, and which in psychology or psychotherapy are termed *basic concepts* because they are planted into the foundation of our life, i.e., our childhood [5, 41]. These concepts are in other words ingrained and deep-seated views on life transmitted to us by our parents or parental figures, teachers or other significant people serving as a life manual from our role models.

What are life concepts? These concepts are the emotional and cognitive constructs that define the way we interpret our relationship with ourselves, with others, and with the environment. It is also a kind of belief about how the world operates and how to deal with it. In other words, they are psychological lenses through which we observe reality and understand our attitude to it. That is why the concepts sometimes sound like a motto or a slogan.

These concepts often serve as motives that guide our behavior, as assimilated norms and habits, which we focus on. Thus, it is not a surprise now that they have such a big impact on our lives. These concepts for quite a long time may not have been made into our consciousness. We are simply not aware of them. This because as children we have no other alternatives and take them for granted. They become noticeable to us just through the clash with the reality that differs from the usual, and where they cease to perform their function, to fulfil the emotional needs and to serve as a reliable guide. When we are children, we do not have much choice in what is offered as facts of life; we simply accept them.

Hence, perhaps it becomes clearer now that not everything that we have experienced will be helpful for us at all times. Some things are a great help and facilitate our lives, whereas others turn it into a nightmare and create a strong potential for future conflicts and problems.

Fortunately, not each family concept will become a conflict in the future. Some concepts are potentially more conflictual than others. For example, the concept "a happy life is possible only if you have a high level of education" is much narrower and potentially more conflictual than the concept, "happiness is possible in any case." Not having the possibility, for different reasons, to obtain higher education, a person may assume that he or she is doomed to misfortune, and the fact of a lack of education will testify that one is unhappy. To make life happier, this person

will have to overcome a lot of doubts related to prior beliefs, but it is better to call them superstitions.

The BC is formed in early childhood through interaction with parental figures and carers, when the satisfaction of our emotional needs (described by N. Peseschkian as I AC) is met with certain conditions (II AC). In other words, when we are no longer simply loved, but loved for something, subject to the fulfillment of secondary norms (conditional love). The child, by accepting the conditions to be loved, internalizes the rules of the family and the corresponding family concepts. In this regard, he is forced to develop certain capabilities of personality (AC), the importance of which, becomes higher. Kirillov (2019), also calls BC a compromise formation, meaning that the child does not receive love in its pure form, but is forced to agree to certain rules. In BC, the child's needs for love, acceptance, trust, tenderness, or hope are exchanged for social norms of the social environment (II Actual Capabilities) like punctuality, orderliness, cleanliness, obedience, politeness, reliability, faithfulness, justice, etc. If I Actual capabilities C are satisfied only on condition of fulfillment of secondary norms (conditional love), Basic Conflict occurs in which the child compromises by accepting the conditions to be loved and thus develops appropriate personality concepts and capabilities. Basic Conflict can be a one-off event, such as the death of a carer. However, more often than not, vulnerability of individual capabilities and possibly limited and conflictual concepts are formed as a result of the accumulation of experiences of conflict or micro-trauma during the developmental process.

Basic concepts are always built around values, and as is well known, values are not always universal, but rather culturally determined. In other words, what is valued in one culture may have absolutely no value in another. This is normal and natural. Now it becomes clearer how many problems we make up and invent by ourselves and thereby artificially create extra difficulties.

Here are some examples of other family concepts:

"You are worth something when you achieve something."
"Partnership is hard work."
"My children have to live better than I do."
"My child should live as well as I lived."
"God is punishing."
"All human beings are good by nature."
"Do not trust anyone."
"You can only trust relatives."
"Guests just bring expenses."
"Compassion would not be enough for everybody."
…etc.

Family concepts that served us very well in our childhood can become a real burden in our adult life, because they stop fulfilling their functions to satisfy our emotional needs. Thus, we can see that family or basic concepts can transform into basic conflicts. Thus, ***the basic conflict is a family concept that becomes dysfunctional because of the current conflictual life situation*** (i.e., because of the actual conflict), and no longer serves to satisfy our emotional needs [16]. In other words, it is a potential Inner Conflict.

Energy that occurs in the actual conflict is multiplied by engaging the content of the basic conflict and leads to the formation of an unconscious inner conflict. Hence, the basic conflict is also imagined as an "awakened dog." For quite a long time this dog was sleeping and did not make any sound. But now, because of the actual conflict, the dog is awakened and lets us experience it by defending the usual values.

The model described below is related to the concepts that operate in the primary family group. In analysis here, we follow two conditions:

1. Concepts should be relevant for socialization.
2. They should concern the relationships with the outside world.

Carriers of these concepts are parents, siblings, grandparents, or people who take over these functions [7].

Operationalization of the Basic Conflict

As well as the actual conflict, the basic conflict needs diagnosis and operationalization. Unlike an actual conflict, the operationalization of a basic conflict is based on two diagnostic aspects:

1. Localization
2. Content

Localization of the Basic Conflict

The basic conflict is described by four concepts each of us carries:

1. Concept of "I"—the attitude of our parents and siblings to us when we were a child.
2. Concept of "You"—the attitudes of our parents to each other when we were a child.
3. Concept of "We"—the attitudes of our parents toward the world around and the social environment when we were a child.
4. Concept of "Primary-We"—the attitudes of our parents toward a worldview, the questions of the meaning of life at the time, when we were a child.

In Fig. 35.15 this is schematically represented.

Thus, these four categories of concepts describe the four dimensions of relationships: attitude toward ourselves, the attitude to partnership, the attitude to society, and the attitude to the worldview. Accordingly, this attitude becomes the starting point for us in the future shaping our own attitude to all these dimensions of relationships. Hence, if the actual conflict is always an external situation, it is about someone; the basic conflict is always about us.

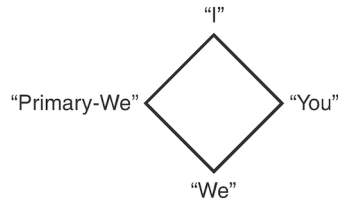


Fig. 35.15 Four dimensions of relationships

The concept "I" contains representations of ourselves, about ourselves, our own values, and their importance. It describes what we encourage ourselves for and what we punish ourselves for. Besides, this concept defines the ways in which we maintain and keep our personal value.

Here are some examples: "Nobody loves me and I don't love myself", "I am a happy person, everything that I undertake turns out. I trust myself", "I always need someone who would help me", "First me, then all the others", "I am a loser. I am an untalented person", "So what, anyway everything is meaningless".

The concept "You" describes our ideas of partnership. It is concerned with what the partnership has to keep it going on, how to function and to be supported. The model for this is the example that was given to us by our parents in their relation to each other.

Examples: "I want to establish the same harmonious family as my parents" or "I will never marry; I won't have children and continue all this rubbish in which my parents are engaged", "Marriage is a burden".

The concept "We" describes the developed relationship to one's surrounding environment and the society. Here, one is concerned with how to perceive it, what to expect, and how to interact with it.

Examples: "Guests are God's gift", "Relatives are like boots, the more one is close to them, the more they squeeze," "You are nothing, your people are everything," "We are on your own and others on their own," "We all are leaves of one branch and fruits of one tree," "It is better to have a rat in a cellar, than relatives in the house."

The concept of the "Primary-We" describes the family ideology, which contains views on the importance of clear life goals and values, religions, philosophies of life, and worldview, that is, the fundamental values that determine the character of life tasks.

Examples: "There is a higher power, merciful and fair," "The World was made comfortable,"

"Live in pleasure, not thinking about death and what will happen after it!"

Hence, all the family concepts, which we find in the client's description, can be distributed among these four categories of concepts. Graphically, it can be expressed as follows:

Concept "I am worth something when I achieve something" is localized in the conception of "I" (Fig. 35.16).

The conception of "Marriage is the work of raising children" describes the idea of partnership value and can be attributed to the conception of "You" (Fig. 35.17).

The concept of "You can only trust relatives" characterizes a friendly social group, in which you can feel safe with and is attributed to the conception of "We" (Fig. 35.18).

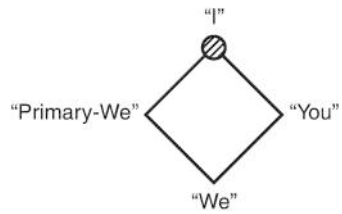


Fig. 35.16 The localization of the basic conflict in the conception of "I"

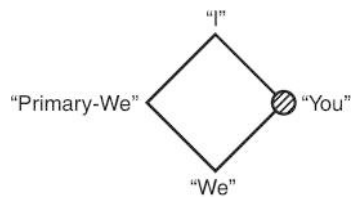


Fig. 35.17 The localization of the basic conflict in the conception of "You"

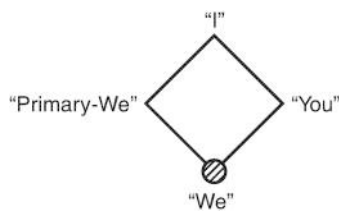


Fig. 35.18 The localization of the basic conflict in the conception of "We"

The conception "children are the main meaning of life" can be assigned to the conception of "Primary-We" and describes the meaningful orientation of life (Fig. 35.19).

Thus, any conception formulated by the client can be attributed to one of the four categories of concepts of the basic conflict.

The Content of the Basic Conflict

The conceptions are always lined up around the values that in PPT are described by the actual capacities. Relationships are described by the primary actual capacities. We understand how we are treated and accepted through the time others spend with us, how others are patient with us, how others are tender with us, etc.

The need for love is a basic need for any human being. We want to be loved and accepted, appreciated, praised, and acknowledged, we want to be given time and some patience for our shortcomings, and many other things. These are our emotional needs. Sometimes, or rather quite often in the process of education, the secondary capacities replace the primary capacities. In such cases love ceases to be unconditional.

It means it exists only because of certain conditions. For example, "my father was patient and gentle to me only if I unquestioningly carried out what he asked me to do, with no arguments" (obedience). Obedience in this case becomes a reliable way of getting some time, tenderness, attention, and care from the father. Another one: "when my mother was angry, my help in cleaning the house could calm her down". Here, orderliness and cleanliness allowed one to get some love, patience, and perhaps contact.

For example, the conception with which we are already familiar "I am worth something when

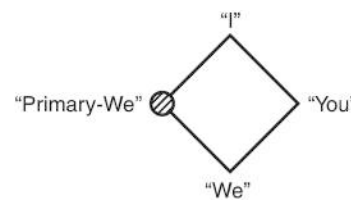


Fig. 35.19 The localization of the basic conflict in the conception of "Primary-We"

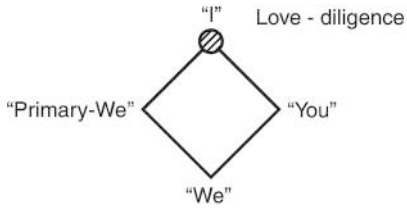


Fig. 35.20 Localization and the content of the basic conflict

I achieve something" is localized in the concept of "I," contains an element of emotional need, expressed through relationship (primary capacity of love) and the social norms, through which this emotional need can be satisfied—the diligence.

It is expressed graphically in Fig. 35.20.

Thus, the content of the basic conflict needs to be described with two actual capacities. One describes an emotional need that is a primary actual capacity. The second is the social norm, through which this emotional need could be satisfied. For example: love—obedience, time—diligence, contact—faithfulness, confidence—reliability. Example: politeness, unity (integrity)—orderliness (order), sexuality—cleanliness, patience—thrift, etc. These are just a few examples of the content of the underlying conflict; actual capacities can be presented in different combinations. If a child is raised in an atmosphere of conditional love, to receive love, one has to fulfill certain conditions: to study well, to help in the home, to be obedient and diligent, etc.

Example: "If I helped my mother to get order in the house, my mother would become kind and soft and would allow me a little bit more" (love—cleanliness, orderliness). Thus, the basic conflict specifies a steady pattern of responses, which in childhood are more or less efficiently served to satisfy the emotional needs of that age. In this case, to gain more emotional warmth from her mother, a client had to fulfill the demands of orderliness and cleanliness.

The secondary capacities serve as social norms of family culture that must be satisfied to meet the emotional and age-specific needs. They are to be described by primary capacities. They allow one to find a creative compromise (including defense mechanisms) between the needs of this developmental period, on the one hand, and adaptation to the social norms on the other and, thus, to get an opportunity to enter the next stage of development.

Inner Conflict

The dominance of one or other motivation may be different at the conscious and unconscious levels [42–44]. For example, a person can consciously intend something but act in accordance with the inducements that are dominant at the unconscious level. In such cases, we are dealing with a disharmonious personality that is being constantly torn apart by inner conflicts. The inner conflict is the result of the addition of psychodynamic forces or conflict dynamics. The ongoing and persistent psychodynamic conflict, in turn, is characterized by an individual's predetermined patterns of experience, which, in the respective situations, lead to continued similar behavior patterns, without the person being aware of them, or being able to overcome them with his or her own free will ("neurotic fixation") [45–47]. Therefore, the inner conflict is also called *neurotic*, emphasizing its dysfunctionality.

A similar conflict arises only under certain conditions. These conditions are simultaneously external and internal. External conditions of the conflict are confined largely to the fact that satisfaction of some deep and active motives and personal relationships become impossible or are threatened. The internal conditions of a psychological conflict are reduced to a contradiction either between different motives and relation-

ships of a person, such as debt and personal interests, or between the capacities and aspirations of the individual. Of course, the internal conditions of a psychological conflict do not arise spontaneously, but rather, in turn, are due to the external situation and history of the individual.

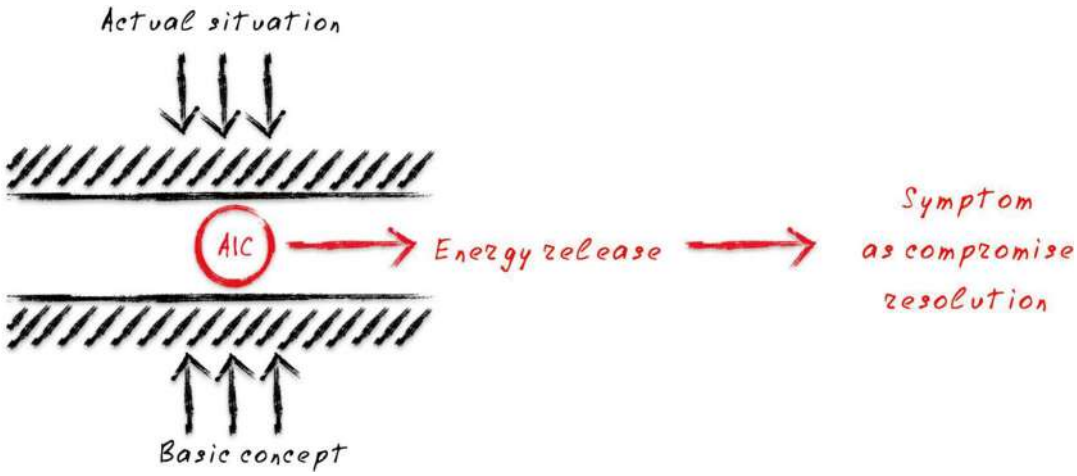
An inner conflict is an unconscious conflict of needs caused by the simultaneous existence of opposing or even mutually exclusive efforts, desires or ideals: for example, the desire to dissolve the marriage owing to unbearable relationships (actual situation) and a moral ban on divorce in virtue of education (basic concept) [16].

Such conditions for so-called acute inner conflict (AIC) or actualized inner conflict.

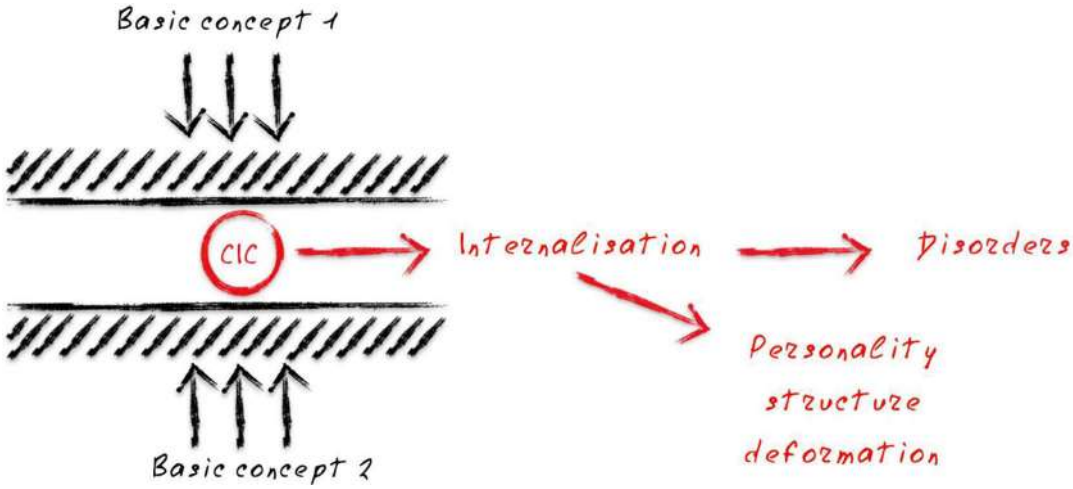
In connection with this the inner conflict is experienced as a temporary state of high emotional tension that feels unsolvable and a deadlocked situation. This dissatisfaction arises from the impossibility of overcoming the problem in a way that was effective for the satisfaction of the emotional needs in childhood. In this conflict, the person repeats his behavioral pattern, no longer works. As a rule, it is a conflict between primary and secondary capacities and has the same content as the basic conflict.

Unlike the above-mentioned actual and basic conflicts, the inner conflict does not look the same and must be described as the basic conflict pattern.

An inner conflict is an unconscious conflict of interest or expectations that are unfolding inside of one personality. It is manifested in repeated dysfunctional conflict patterns of the basic conflict. Recurrent conflict patterns of the basic conflict no longer resolve the conflict, but just create additional tension in the inner conflict.



However, there are also so-called life-defining internalized or intrapsychic conflicts, described in detail in the work of Operationalized Psychodynamic Diagnosis-2 (OPD-2). If AIC is "I can't do this now." Chronic Internal Conflict (CIC) is "I couldn't do this ever". CIC has a timing of conflict. It is associated with the onset of a new stressful actual situation causing AC.



In CIC, the conflict-free period is difficult to define. Prolonged existence of conflict since childhood leads to its deep internalization. A person has practically no experience of life outside the conflict.

In such a scenario of IC development, the simultaneous existence of contradictory family concepts from childhood leads to its chronic form, which leads to deformation of character or personality. For example, the prohibition against boasting: "Pride is a sin" and "everything you do, you must do well". Such a combination of concepts is initially very conflictual, because on the one hand, condemns its bearer to do everything perfectly, on the other hand, to be blamed in every possible way for impulses to self-encouragement or depreciation of encouragement from the outside. The person lives in a constant conflict of guilt, shame, over-control, identity, etc.

Thus, the conflict remains unresolved and is internalized along with conflict-related negative reactions such as fear, anger, shame, and guilt. The possible unconscious emergency solution of the child's soul becomes the suppression of the impulse or need along with the accompanying fear and other negative reactions of consciousness.

Clinical Case Example Describing the Four Categories of Conflict

The actual situation: a woman, 35 years old, recently experienced a worsening of her relationship with her husband. They began quarreling more frequently over trifles, mainly on the subject of justice, who and what kind of responsibilities are taken in their family. Over a period of time, they started to communicate less. The client

began to experience a deficiency of attention and time from her husband. The husband began to spend more time at work, whereas the client was more involved in the housework. She felt lonely and that her husband loved her less.

The basic situation: in childhood, the client's mother was occupied by the household most of the time; she was quite cold and an emotionally greedy woman. She never had any free time, for example, to play with her daughter. To receive some warmth, some time, and acceptance from the mother, the client had to clean the house or help her mother with the housework. It made her mother happier, and she would become emotionally warmer and softer; thus, the daughter experienced the feeling of acceptance and love.

Inner conflict: now, at the present time, when the client feels a lack of love from her husband, she unconsciously reproduces the pattern of the basic conflict (love—cleanliness, orderliness). When her husband comes home, she begins to clean the house. Now, she has no time for her husband. The husband sees that his wife is occupied by the vigorous cleaning and goes to his room. The wife, in turn, seeing that her husband has again not encouraged her by paying her attention, feels rejected and escapes into work even more actively (Fig. 35.21). After an hour she experienced a hypertensive crisis [16].

What happened? The client's actual conflict is localized in the domain of contacts and occurs in her relationship with the husband. In content, the

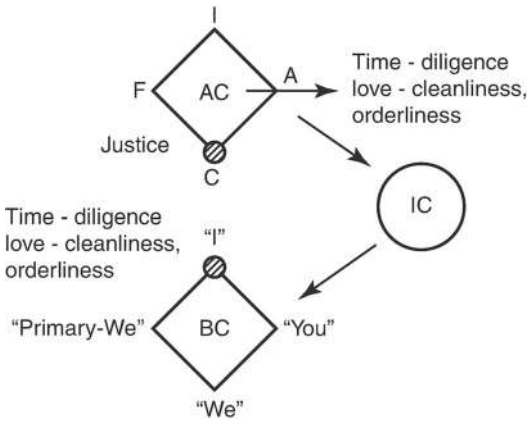


Fig. 35.21 The scheme of inner conflict occurring

actual conflict is linked to the theme of justice. She expects greater justice, expressed in anticipation of her husband's help in taking care of the house. For example, the client believes that her husband must help her clean the house, and the husband thinks that this is not necessary: "it is already clean enough" or "I've been working so hard the whole day." However, the views and expectations of the client about justice are not met. Although her conflict is processed by "escaping into activities" at home, her husband deals with the same conflict by "escaping into activities" at work. The result is the same; the conflict in this relationship is not solved because the necessary energy is not invested into it.

One of the client's basic concepts is "time and love must be earned." In this sense, love is perceived rather as something that does not come just like that, that must be earned, and looks more like justice. This explains the client's special sensitivity to the topic of justice. To earn love, time, and attention from the mother, she was able to succeed with the help of cleanliness, orderliness, and diligence. Only in response to these norms could the mother satisfy an emotional need for the affection of the daughter. Thus, she also manages to maintain her self-worth through these social values. Now, when the client feels devoid of love, her need for love has been actualized, which she is unconsciously trying to "earn," referring to the pattern of the basic concept. Here is the point where the basic concept is becoming a basic conflict as it does not execute the functions it had in childhood: to satisfy an emotional need for love. The client repeatedly refers to the usual pattern, which turns out to be inefficient in the current problematic situation. An inner conflict has been created with the growing tension. Here, the symptom of high blood pressure appears, which performs the function of a temporary compromise resolution. Now, the situation is changing. The husband leaves work and gives more time and attention to his wife. Temporarily, the client may not revert to the usual pattern, because this function is taken up by the symptom. The conflict temporarily loses its relevance owing to the emergence of a new actual situation (Fig. 35.22).

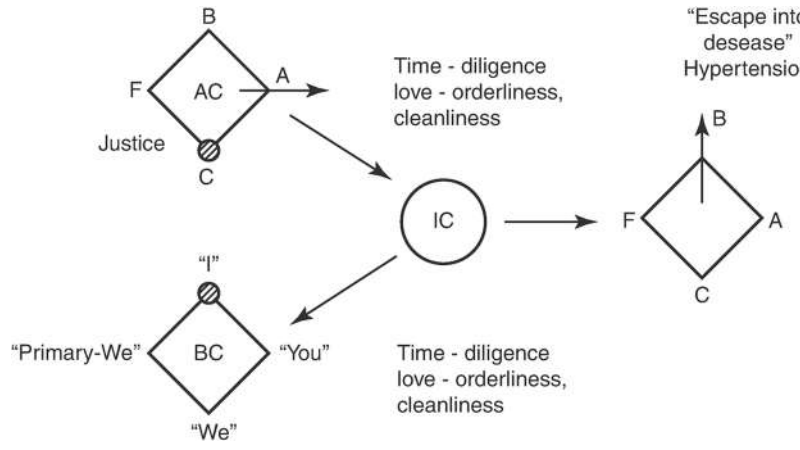


Fig. 35.22 The scheme of the appearance of the conflict dynamic with symptom formation

Psychodynamic conflicts are thus internal, unconscious conflicts and must be delineated from external or internal stressful conflictual demands. Internal, unconscious conflicts play a decisive role in the origin of mental and psychosomatic disorders. Unconscious intrapsychic conflicts are unconscious internal clashes of diametrically opposite motivation sets, such as the basic needs to receive care and to be self-sufficient [48–50].

The tension of an internal conflict is maintained until the actual conflict is solved or the basic concept is revised.

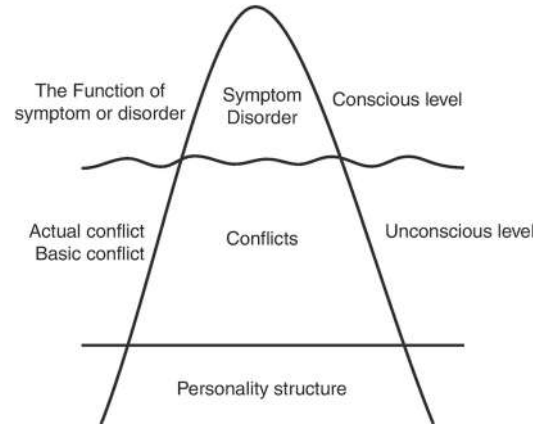


Fig. 35.23 Iceberg model

Interaction and Interrelation of the Conflicts in Positive Psychotherapy

The work with conflicts can absolutely and reasonably be defined as a goal of psychotherapy. For a better understanding of the formation of conflict dynamics, a good understanding of how the various conflicts interact with each other is required. After Sigmund Freud discovered unconsciousness and created his own model of the psychic apparatus, the "level" of awareness became firmly rooted in psychodynamic psychotherapy and later repeatedly compared to an iceberg. There even appeared a so-called "iceberg model," which illustrates the surface and underwater parts of psychological experience.

As is known, not all conflicts are available for awareness. The upper part is much smaller than the lower part and illustrates the observed or conscious content; for example, symptoms and disorders. The actual conflicts, as is also known, are more often conscious, but sometimes they hide in a "shallow depth." However, it is not very difficult to detect. More deeply located are the unconscious conflicts and structural capacity. This is graphically depicted in Fig. 35.23.

The actual conflict is more often conscious, but not always so; therefore, it is mobile and is located on the border of consciousness. It can be reasonably compared with a fishing float bobbing in the water. When the actual conflict "clings" to

the basic conflict, it goes under the water, becoming less accessible for awareness or even becomes unconscious. The topographical location of conflicts in relation to each other and levels of awareness are portrayed in Fig. 35.24.

The Key conflict as a derivative of the actual conflict is conscious, but the inner conflict, as derived from the interactions of the actual and basic conflicts, is unconscious (Fig. 35.25).

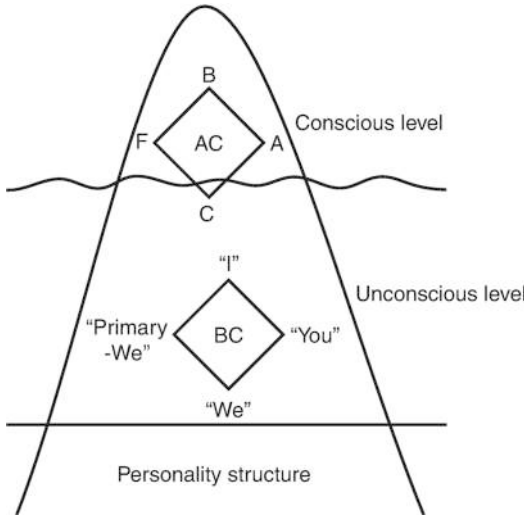


Fig. 35.24 Topographical model of conflicts in positive psychotherapy

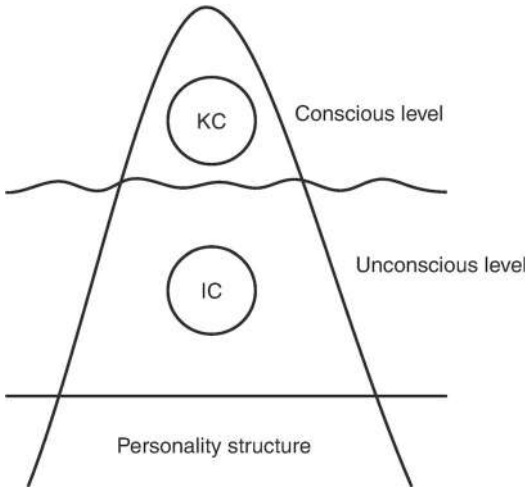


Fig. 35.25 Topographical model of the key and inner conflicts in positive psychotherapy

The Actual Conflict and the Key Conflict

In fact, the key conflict is a derivative of the actual conflict because it does not exist away from it. The key conflict processes the actual conflict, directing the conflict energy outside or inside. Both conflicts are conscious and displayed in relationships with other people. During the psychotherapeutic process, we teach our client to understand his/her conflicts and the habitual ways of reacting to them. We help him/her to learn how to constructively share their experience, maintaining the balance of honesty and courtesy, and how to build a fair and open consultation and effective dialogue.

The Basic Conflict and Key Conflict

Here, we can talk about the level of self-discovery and increase in self-awareness, which can be achieved through understanding of the family concepts related to family traditions. Said simply, the unconscious experience cannot be verbalized. In the process of psychotherapy and with the help of a therapist, the client's awareness usually increases and, accordingly, this increases his/her ability to verbalize the findings. However, this is possible independently as well, when the client already has the skill to work through it by himself. Furthermore, family concepts can also be structured around the values of honesty and courtesy, and thereby affect their manifestation in conflict handling.

The Inner Conflict and Key Conflict

Both of these conflicts are similar in that they unfold within one person. However, unlike the inner conflict, the key conflict is conscious. The inner conflict is a derivative of the actual and basic conflicts. Verbalization of an inner conflict is possible only after awareness, or its possible resolution. This usually occurs during psychotherapy after analysis of all components of the conflict dynamics. Only then does the ability to verbalize it consciously appear.

Actual Capacities and the Key Conflict

Actual capacities as cultural values are contained in all of our conflicts. Being social values, they are very active psychodynamic units. Here, a person's ability to speak out about the actual capacities may differ substantially. For example, talking about thrift, faith or sexuality is more difficult than, for example, talking about punctuality or orderliness. This, of course, depends on the cultural values of these abilities.

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36

Using Stories, Anecdotes, and Humor in Positive Psychotherapy

Olga Lytvynenko, Farid Peseschkian, Lyudmila Zlatova, Volodymyr Karikash, and Tetiana Zhumatii

Introduction

> *For centuries, stories and metaphors have been used to convey a message. Religious books are filled with analogies, metaphors and stories … They all convey teachings in an indirect way, to translate difficult ideas into tangible ones. The wise men and women of villages have used metaphors and stories to answer a question or explain a difficult matter for centuries. They were part of an oral culture in which the elderly had an opportunity to tell stories and share wisdom.* (Rowshan [27])

Every engaging story, from brief anecdotes to extensive novels, cherished children's fables, or even street advertisements, seeks to forge an emotional connection with its characters. The power of storytelling lies in our ability to empathize with figures from realms unlike our own. This distinctive human capacity to invent and immerse ourselves in fiction has played a crucial role in shaping the complex societies we inhabit today. Without the power of belief, commerce, nations, and basically any structure surpassing the population of a small tribe, would not be possible. It is believed that this ability started with the cognitive revolution about 70,000 years ago with stories oftentimes portraying the unexplainable like natural disasters, the world and its origin, but also more rudimentary acts like the memory of great hunts. Through these tales, elders passed on their wisdom and shaped the beliefs and behavior of a whole culture, generation upon generation thereby enabling us to become creators of our world.

Closely related to the art of storytelling has always been the healing profession. Before the beginnings of the scientific method, shamans, medicine men, or quacks were consulted for diseases. Oftentimes, they took advantage of the effect that stories have on people and spoke of miracles and tales of salvation—presumably to emphasize the effect of the "medication."

There are countless examples of how stories were used as a life aid, in the broadest sense even specifically as psychotherapy, long before psychotherapy became a scientific discipline. Probably the best known of these examples is the Arabic story collection *One Thousand and One Nights*, which tells of the healing of a mentally ill

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_36

monarch with the help of stories. Shahriyar, king of an unnamed island 'between India and the Empire of China', is so shocked and enraged by his wife's infidelity that he has her killed and gives orders that henceforth a new virgin will be brought to him every night, each of whom will also be killed the next morning. After some time, Scheherazade wants to stop the killing and begins to tell him stories; at the end of the night, she has reached such an exciting part that the king is desperate to hear the continuation and postpones the execution. The following night, Scheherazade continues telling the story, pauses again in the morning at an exciting point, and so on. After a thousand and one nights, the king grants her mercy, marries her, and ends the executions. Through the power of stories, the afflicted king was able to overcome his thirst for revenge and embody humanity again.

This story may be examined through two different lenses: firstly, in the broadest sense, relief is achieved by successful treatment of a heartbroken, mad king by an intelligent, resourceful woman; secondly, through individual narratives, Scheherazade's tales serve as treatment for suffering readers, who absorb their content, draw lessons from them and apply those stories to their lives.

With the development of psychotherapy, storytelling also found its way into the scientific realm. Various therapy methods incorporate stories of different types and origins, myths, fairy tales, legends, metaphors, and proverbs, into their approaches to improve the treatment of patients. Stories were used by different psychotherapeutic schools for their various effects, such as being less invasive than directional talking and thus easier to accept by the patient [17], or that they encourage patients to use their fantasy and imagination and thereby aiding them in connecting with their inner children [24]. Further, stories are beneficial to psychotherapy because of their emotional value, they will be remembered longer than conventional advice [2, 4, 16].

Background Positive Psychotherapy (PPT after N. Peseschkian) leverages storytelling to enhance therapeutic processes. In this approach, treatment extends beyond the traditional bounds of the professional-patient dynamic, incorporating self-help techniques and preventative education. Central to PPT is the 'Positive Approach', which perceives patients not merely as carriers of ailments but as individuals endowed with the capacity for healing. The therapist's role is to bolster these intrinsic strengths, utilizing narratives as powerful tools in the journey to well-being. "If the doctor tells good stories to his patients, he only needs half as much anesthesia." (Sauerbruch, cited in N. Peseschkian [23]).

Positive Psychotherapy, which incorporated the conceptual ideas of other modalities (psychoanalysis, analytical psychology, cognitive-behavioral psychotherapy, humanistic approach, systemic psychotherapy, etc.), in many respects uses the appropriate methods and techniques of working with metaphors. At the same time, there are some differences and peculiarities in the Positive Psychotherapy approach. Next to the positive approach, the PPT's peculiarity is the **transcultural approach** with the main question: *What do we have in common and what is our difference?* In this context, the metaphors of their culture may play the role of forming a common emotional-semantic therapeutic field of contact, and the metaphors of other cultures may have the effect of novelty and carry counter concepts of new experience [22].

In PPT stories are further used for their transcultural aspects and possibilities as each story offers both solutions and problems common in the specific cultural context. Stories from foreign cultures provide information concerning the rules of life and concepts considered important there, show other models of thinking, and make it possible to question and expand one's own repertoire of concepts, values, and conflict resolution skills [24]. In this way, stories can be used to break down emotional barriers, prejudices, and resentments against foreign ways of thinking and feeling, which make the foreign perceived as something aggressive and threatening, producing resistance where understanding would be adequate. Seen in this light, stories can be a decisive approach to a fundamental change in conscious-

ness, which in turn is a prerequisite for change in politics, economics, and behavior towards our natural and social environment [25].

Nossrat Peseschkian was one of the first psychotherapists to use parables and humor in working with patients. He managed to see and appreciate the simple and understandable stories and successfully use them in therapy. His book "The Merchant and the Parrot – Oriental Stories as Tools in Positive Psychotherapy" became a classic work and is quoted by representatives of various psychotherapeutic areas [23]. Speaking about the therapeutic properties of stories and parables, pointing out that they are used by the patient as metaphors, and also considering emerging experiences, emotions, and insights, Nossrat Peseschkian introduces the notion of "history function" into a scientific discussion [23]. These functions are: mirror, model, mediator, repository, transmitter of tradition, aid to regression, and serving as counter-points.

The Mirror Function. The content of the parable becomes the mirror that reflects the inner world of the person, thus facilitating identification with him.

The Model Function. The parables reflect various conflict situations and suggest possible ways to solve them, or indicate the consequences of individual attempts to resolve conflicts. Thus, they help to learn using the model.

The Mediator Function. The parable acts as a mediator between the patient and the therapist, thereby reducing the patient's resistance.

The Repository Effect (storage of experience). After the end of psychotherapeutic work, parables continue to operate in a person's daily life.

Stories as Transmitters of Tradition. Describing individual situations, the stories take them beyond the limits of individual experience and appeal to the cultural traditions and experiences of different generations.

Stories as an Aid to Regression. The parable helps the patient to return to the former joyful immediacy. It causes surprise and bewilderment, opening access to the world of fantasy, figurative thinking, direct and no one condemned entry into the role that content offers them.

Stories as Counter-Concepts. The parable does not sound to the patient in the generally accepted, predetermined sense, but suggests to him "... an alternative concept that the patient can either accept or reject. Stories are only a special case of human communication, involving the exchange of concepts." [23].

Change of Perspective. Proverbs unexpectedly cause a new experience for the patient, a change in attitude occurs in his consciousness [23].

In the second chapter of the book "The Merchant and the Parrot," Nossrat Peseschkian describes real cases from his practice using stories in various therapeutic situations. Comments to the parables proposed by the author can serve as a guide for the work of practicing psychologists and psychotherapists. The "patient-story-therapist" method of interaction proposed by Nossrat Peseschkian has been successfully incorporated into Positive Psychotherapy.

Development of the Method, Current State

In recent decades, the scientific and practical development of PPT has become increasingly intensive, with many authors contributing to the understanding of therapeutic work with stories and humor. One prominent author continuing the traditions of N. Peseschkian is Arno Remmers, who asserts that "Stories are association aids that lead to the unconscious core." [26].

Remmers expands conceptually on Peseschkian's functions and modernizes them through his professional experience and contemporary scientific research. His work involves:

- Working with stories: narrative psychotherapy. He describes the use of narratives in psychotherapy as a process involving stages of identification and development.
- Practical recommendations on when and at which stage of the five-step model of

psychotherapy to use different stories, depending on the goals and needs of the patients.
– Work with dreams. Dreams as personal stories and reflections of an individual's myth.
– Descriptions of his practical experience and specific techniques used in his therapeutic work.

Remmers states: "The characteristic of a narrative psychodynamic method is shown clearly here: Stories are aids in association which lead the way to the deeper, unconscious core. Within a five-step process of interaction narrative approaches induce changes of perspectives, belief systems, and defense mechanisms in therapy" [26].

A new perspective on the understanding of the function of stories was added by a positive psychotherapist from Poland, Klaudia Kolenda-Sujecka, in her work "Komunikacja metaforyczna" [15]. In this work she expanded the understanding of the traditional functions of stories described by N. Peseschkian by increasing their content continuum through active and passive modus. Table 36.1 describes this contin-

Table 36.1 Description of the function of stories in the dimensions: "to" and "from" the patient [15]

Function name	Active dimension	Passive dimension
1. Mirror function	By sharing stories and metaphors with the therapist, the patient gives expression to his inner IAM, unconscious conflicts and deeply hidden feelings. E.g., "*I feel trapped.*"	The story offered by the therapist allows you to take a safe look at those areas of your own personality that are inaccessible to the conscious experience.
2. Function of the model	The story brought by the patient relates to the model of relationship he presents with himself and the environment.	The stories reflect daily conflicts and offer solutions previously unavailable to the patient's consciousness and new coping strategies in the difficult situations.
3. Mediator function	The patient may share metaphorically such aspects of his or her own life that, due to perceived shame, guilt, anger or fear, he or she would not dare to bring up directly.	With the help of a story, the therapist reaches the part of the patient's experience that is covered by defense mechanisms and arouses resistance, when trying to gain insight. Talking about the story's protagonist, who is experiencing difficulties identical to the patient's, allows the patient to safely discuss their own conflicts.
4. Function prolonged action	The stories about their own lives that the patient shares are often so vivid that they experience them as if they	By engaging the patient's imagination and emotions, the action of the story persists in the patient's memory even
5. Function of the tradition carrier	The patient, sharing in a metaphorical way various contents from his life (anecdotes, stories, metaphors), at the same time invites us to get to know the world of his family, cultural, national traditions, emphasizing those values that are for him the most relevant.	The metaphors suggested by the therapist may refer to values that are beneficial for the development of an individual in a certain culture, and which the patient has not had access to before, e.g., trust, reliability, punctuality.
6. Function intermediary between cultures	Patients also bring their own stories, which they find surprising, incomprehensible or strongly emotional. These may relate to transcultural differences at the micro- and macro-level. Discussing such stories together, taking into account both differences and similarities, increases a sense of understanding and is helpful in resolving intrapsychic conflicts and interpersonal.	Stories coming from another culture, discover new ways for the patient to cope, stimulate the development of potentialities and tame what is "other" and "new." Ultimately, this reduces anxiety and lowers the level of mutual hostility.

(continued)

Table 36.1 (continued)

Function name	Active dimension	Passive dimension
7. Function of the regression tool	Through the medium of childhood stories, the patient can invite the therapist to join in the "play" by discovering his childhood nature and the symbolic language associated with it (magical thinking)	The content of the stories constructed by the therapist is addressed to the patient's childhood intuition and imagination. This allows him to go back to earlier stages of development, where there is more space for spontaneity and finding pleasure in play.
8. Function alternative worldview	The stories brought in by patients can sometimes contradict their main narrative and offer alternative worldviews regarding their personal experiences. For example, a patient who maintains that he is not good for anything may contribute a story about the pride his parents felt when they saw him perform in the school theater.	Using storytelling, the therapist does not impose the only right theory on the patient, but presents him with an alternative worldview that he can accept or reject.
9. Function changes prospects	Sharing the patient's story, which causes him to surprise or stir, serves as a means to write a new narrative of one's own life and the replacement of the old knowledge of self and the world to a new and more adapted to its current realities of life.	The stories are more than just a description of the various events arranged and are integrated into a coherent narrative. Introducing an event that is surprising and not matching the whole, the therapist can urge the patient to new surprising insight or even glares.

uum while suggesting possibilities for the therapeutic use of stories in different cases.

Next, Kolenda-Sujecka, analyzing the latest research in neuroscience about how the use of stories in working with another person affects brain activity, which is reflected in the functioning of the human body and mind, connects this to practical experience. This enabled her to highlight the following additional functions of stories:

1. Using the stories affects the functioning of the endocrine system and consequently social relations.
2. Stories can stimulate the occurrence of specific emotions and behaviors.
3. Using stories helps to activate the whole potential of our brain and the integration of new knowledge within old neural circuits.
4. Stories help us to remember what is important [14].

Thus, impressive scientific neurobiological findings confirm the practical experience of many psychotherapeutic schools and allow us to integrate human and biological knowledge into a holistic view of the nature of the impact of stories on human beings. Developing this holistic view, Klaudia also introduces the concept of ***non-directive communication,*** which means "a form of therapeutic intervention using an anecdote, metaphor, parable, fable, legend, or fairy tale that addresses the unconscious part of the patient's psyche in order to stimulate non-linear thinking. She also states that "verbal messages in metaphorical form are usually addressed to our unconscious, in particular to its intuitive, imaginative or symbolic faculties. Therefore, we say that this way of communicating is non-directive and involves activation of right-hemispheric processes in the brain, in contrast to directive communication, which stimulates left-hemispheric processes" (Kolenda-Sujecka [15]).

Currently, we are also seeing intensive practical development of a wide variety of techniques and ways of working with stories in Positive Psychotherapy. Werringloer [28] is developing such a direction as educational psychology; Lytvynenko [19, 20] has proposed a practical model of working in five stages with fairy tales and movie stories; Bencheva [3] is looking for opportunities to find Positum and reach the hid-

den abilities of a person with the help of fairy tale stories—the function of a mirror; Zlatova [29] investigated the role of anecdotes in the work of a psychotherapist and highlighted the functions of stories-jokes; Messias [21] suggests using artistic poetic works in the psychotherapeutic process.

Ivanova [10] makes her contribution to the development of ways of working with stories, she describes practical transcultural aspects of using Bulgarian fairy tales in a contemporary Bulgarian environment when working with women with sexual trauma; in another paper Ivanova [11] describes clinical work with children using fairy tales from world culture: "In the form of a collective image, seven fairy tales, seven stories, and seven archetypal images of child suffering are presented. Suffering clothed in a story is something that can be lived with even in the most difficult environment."

A new book on psychotherapeutic work with stories has been written by F. Peseschkian and Torres [25]. The authors continue the tradition of therapeutic storytelling using a new "lexicon approach." An overview of the scientific basis of therapeutic storytelling is followed by a collection of anecdotes and aphorisms with keywords describing disorders and situations that may be relevant to people for whom a particular entry may be helpful. The book includes a large number of stories, parables, and anecdotes that can be used in the therapeutic process for clients and patients with a wide variety of challenges and needs, from some aspect of mental illness to challenging life circumstances. The authors suggest that their contribution to the field may lead to a different experience of talk therapy and offer a description of the characteristics of good storytelling.

Jaźwińska-Chren [12] draws attention to the specifics in working with written and oral stories. The author connects with anthropology and describes the influence of live encounters, voice, intonation, and the possibilities that emerge in working with the perception of a written story. Jaźwińska-Chren offers an exercise to understand what is best for the client, and what way of working with stories is needed. She also offers insights into why the client prefers to work with oral or written versions and relates the client's choice to the main ways of processing according to the balance model. One practice option the author suggests is to write a story for a client. "Storytelling serves as a multifaceted tool for personal development, enabling individuals to explore their inner world, connect with others, and define their identity through the power of narrative."

So, the current stage of Positive Psychotherapy shows the dynamic development of this direction, stories and humor arouse a lively human interest in both clients and psychotherapists. Colleagues reinforce their personal discoveries with the latest neuroscience. This scientific and practical experience proves the effectiveness of using stories to address a wide variety of therapeutic challenges. Some traditional and new methods and techniques for the practical application of stories of different types will be described below.

Clinical Applications

There are multiple ways in which incorporated stories can positively impact therapy outcomes. In the following, we take a look at some of the most important ones.

Case Example: The Traditional Use of Stories

A 43-year-old patient came to a psychotherapist, in a state of acute grief. Her only 21-year-old son has committed suicide. At that time, the woman was divorced for a long time. As a result of the tragic death of her son, she was left all alone and constantly thought about her dead child. All its meanings have been lost. She did not touch her son's things in his room and was in the process of denying her loss. After 3 months of therapy, when the intensity of emotional tension decreased slightly, the therapist resorted to the Persian parable of the Glass Sarcophagus.

Glass Sarcophagus

An oriental king loved his enchanting, beautiful wife above all else and sought her presence day and night. Unexpectedly, the king's darling

passed away, casting him into the depths of overwhelming grief. "Never," he exclaimed, "will I part from my beloved, even though death has taken her." He laid his deceased wife in a glass sarcophagus, which he placed in a great hall, his bed beside it. The proximity of his wife's body was the only comfort left to him.

Though the palace was cool, the dead woman's remains were slowly decomposing. Her lovely face changed color and became increasingly bloated as each day passed. Consumed by love, the king was oblivious to the course of nature. Soon the foul smell of decay filled the hall and no servant could bear to enter. Although the windows were wide open, the odor of decay crept up on him, which no amount of rose oil could mask. Finally, he tied his green sash, the emblem of his royal lineage, over his nose. Nothing helped. The servants and courtiers fled and the only companions of the grieving king were buzzing black flies.

Shortly thereafter, the king lost consciousness. His physician, the hakim, had him removed to the palace garden. When the king awoke, a fresh breeze swept over him. The scent of roses revived him and the splash of fountains gladdened his heart. He imagined the love of his life was still alive. After a few days in the garden, the king's health was restored and he began to come alive again. Looking into the calyx of a rose, the king suddenly recalled his wife's beauty and became sickened by his clinging to a decaying corpse. He picked the rose, placed it on the sarcophagus and ordered his servants to consign his wife's remains to the earth. (from: N. Peseschkian, Oriental Stories, [23])

The patient was silent for a while and then said that the same sarcophagus was "standing" at her apartment. This parable acted as a "mirror" and led it out of the stage of denial. In further sessions, returning to the image of the "glass sarcophagus," both the therapist and the patient understood well what was going on. Thus, this parable also performed the "prolongation" function and became an intermediary for further work.

The traditional use of parables and stories is still popular and frequent among positive psychotherapists. The same parable causes unique living and insights for different patients. At the same time, PPT is a dynamically developing method in which the traditional techniques of working with a patient are closely intertwined with new ways and integrating capabilities from adjacent modalities. Psychotherapists can also use various versions of stories in their work—in a situation of situational encouragement to relieve the patient's tension and anxiety with humor. In advanced cases, the patient can independently write a story or a fairy tale that will help uncover a subjective view of his life situation, limitations, abilities, and resources. Sometimes it will be useful to offer the patient to write the most dramatic and optimistic end of a story or a fairy tale. In any case, the use of metaphors allows using three vectors of the patient's subjective reality: mystical, cognitive, and reflexive. At the same time, it is possible to operate freely with three times—past, present, and future—depending on the therapeutic tasks [13].

Use of Fairy Tales in Therapy

Psychology has always shown interest in the study of fairy tales and myths. The classics of psychology have repeatedly turned to the analysis of fairy tales. Freud himself believed that in myths and fairy tales, in folk sayings and songs, in common usage and poetic fantasy, the same symbols are used that allow interpreting dreams [8].

There are different ways of working with a fairy tale as a tool for help and development in Positive Psychotherapy: using a fairy tale as a metaphor, drawing based on a fairy tale, discussing behavior, motives, plot, psychodrama, playing of the whole fairy tale or individual episodes. It is also possible to use a fairy tale as a parable or creative work, which in turn includes analysis, narration, rewriting, and the creation of a personal fairy tale as a self-fulfilling prophecy with its plots and heroes.

Remmers focuses on working with fairy stories in the psychotherapy of children and adolescents. He states: "The Grimm fairy tales of the

German speaking culture make use of the same motifs that occur in dreams and in developmental tasks: "Hansel and Gretel" or "Mother Holle" as cruel as these fairy tales may seem to adults, they symbolize the natural process of detachment up to the self-responsible decision, action, and self-sufficiency. Fairy tales create a logical film and role-play in the child, they initiate communal experience and exchange, trigger feelings in the narrative situation, and captivate the listener" [26]. The author suggests using fairy tale work to diagnose the psychodynamics of the child and utilizes the therapist and his young patient to co-create a fairy tale. "I begin as a therapist with "Once upon a time there was a prince who..." and the child continues the story until it passes the narrative thread back to me, just as I then pass it on to the child again until it has become a work of its own with person, themes, drama, and dissolution. In the narrative that emerges interactively—sometimes interrupted by laughter or amazement—the child's central themes emerge that might otherwise be seen in the play, but are often unconscious to the family." [26].

The characters created by the patient in a metaphorical form reflect his own psychological processes and can then be felt, lived, and analyzed in an environmentally friendly form of identification and self-reflection, and also gives a person the resources to adapt [18, 19].

Working with a fairy tale, as well as working with a parable, is widely used as a narrative for both diagnosis and correction. By listening to and perceiving fairy tales, a person identifies with the characters, embeds fairy tales into his life scenario, and forms it. And, of course, as an opportunity to clarify internal conflicts, and unconscious motives, to establish a connection between the conscious and the unconscious.

The main purpose of using fairy tales in Positive Psychotherapy is through personal experience and living-learning the basic principles of PPT, finding resources, finding new strategies, the possibility of successful socialization, adaptation, and the ability to respond to life challenges.

Writing a fairy tale as part of individual or family counseling fits in well with the five-step strategy of Positive Psychotherapy. The stage of counseling and the stage of the fairytale narration relate and lead the client to a "fairytale" solution to the problem, which helps not only to structure the process but also to achieve a therapeutic result [19].

Stage 1: Observation, we suggest starting a fairy tale with some traditional beginnings. The patient describes the main character and tells about his life situation at the time of the beginning of the fairy tale (actual situation).

Stage 2: Inventory. At this stage, the fairy tale metaphorically describes the challenge, symptom, or actual conflict that occurred in the life of the patient. Formulation of the request.

Stage 3: Situational Encouragement. What has changed in the life of the patient after the event, the challenge? How did he cope, what skills did he show?

Stage 4: Verbalization. At this fabulous stage, the client describes the development of events, suggests ways out, looks for magic helpers and magical objects, overcomes all difficulties, and completes the Hero's Journey.

Stage 5: Broadening of Goals. At this stage, the patient describes the outcome, to which the hero came as a result of his or her tests, what conclusions he made, and what he brings with him further into his life.

At the end of the tale, it is possible to offer patients a metaphorical delineation of the moral of the whole tale, to understand what symbolic function and task the symptom, and conflict in the life of the patient performs. After writing and reading—discussing the patient's feelings about his fairy tale, analyzing the motives and behavior of the characters, of necessity—writing and rewriting the fairy tale [19].

The fairy tale analysis in Positive Psychotherapy allows not only to work with the **actual** and **key conflict** but also to detect the **basic** and **internal conflict** of the patient. In the logic of building a fairy-tale plot, family scenarios, concepts, and beliefs, including intense emotional experiences, are clearly visible.

Case Example: Psychosomatic Symptoms

Stage 1: ***Observation and Distancing***. The patient is a woman of 31 years old, married, and has a child. She came with a request to get rid of the rash. There was a rash as a symptom 6 months ago. It periodically appears and disappears on the body. If the patient is nervous, then begins to scratch the body and a red rash appears on the skin. One of the most pronounced **actual capacities** of a woman is obedience, as well as responsibility, punctuality. When in her life there was a question of choosing a new job, perhaps a change in her profession and, in connection with this, a new way of life in the family, the woman began to get nervous and notice irritation on her skin.

Stage 2: ***Inventory***. Asking clarifying questions, it was possible to find out that the patient very much doubts her abilities and opportunities to find work. One of her family concepts is "Nothing gets easy in this life. It takes a lot of work." Fear of failing a new business and disappointing loved ones paralyzed the woman and prevented her from moving forward.

Stage 3: ***Situational Encouragement***. An important point in the work was to identify the strengths of the patient, such as patience, diligence, accuracy, and good faith. Attempts to be perfect in any business caused the woman's anxiety, therefore one of the tasks was to draw her attention to what the patient had already achieved in her life and learn to value herself and her achievements.

Stage 4: ***Verbalization***. At this stage, the patient was asked to write a fairy tale in which she could metaphorically identify her symptom and reflect on the "magic" resolution of the situation.

The tale about psychosomatic symptoms.

Once upon a time, there was Thumbelina. Little, smart girl, loved by her parents. She loved to wear purple things. So she was comfortable. She also loved to read different books and go into this world of fantasy, to experience events with the characters. Everything was beautiful in her world. For the time being ...

Then one day, walking on a pleasant summer evening, she met Cerseya. A terrible, disgusting, worse than the most vile toad, all in abscesses, she appeared on the way at Thumbelina, still smiling, but this did not look more pleasant. The first thing Thumbelina felt was horror! This shameless pimply lady said hello to her, calling her name, and hugged her. "Hello, my girl. Hello Thumbelina! I see you are sad and lonely ... Me too. You and I are so alike. Let's go for a walk together."

Thumbelina did not know what to answer. She didn't want such a friend, but she didn't have any others, and one cannot be judged by appearance either. We must first talk, and suddenly this acquaintance will be useful.

"Come on," she replied. – Let's go."

"I look sad to you."

"Depression ... – Thumbelina said for some reason".

And Cerseya took advantage of this phrase, word for word, and plunged into her confidence. Got into the most secret corners of her soul, and took all the space around. Because of this, Thumbelina's life has become even more isolated. Nobody approached her, did not talk, and did not look at her. She was not attracted to communicating with her rash. And Cerseya reassured her in her own way. "See, you don't need it! Absolutely! Only me. I am your true friend. I love and appreciate you." And Thumbelina had no choice but to believe it. And my soul was getting harder.

And then, somehow, we had a nice summer day, at the regular walk of two inseparable friends in misfortune, the Wizard appeared. Beautiful with intelligent lively eyes, and most importantly, with a genuine interest in Thumbelina. For the first time in many years! He came to meet and suggested that she go on a great balloon flight. Of course, Thumbelina became interested and was "in favor" with both hands. But Cerseya does not sleep. She also wanted to fly with the Wizard, she must be there!

Then the Wizard suggested Cerseya go to the country of the same "beauties". Moreover, gratu-

itously, in a beautiful paradise. Cerseya was delighted, and immediately forgot about her friend and devoted friendship. She jumped into the ball and was like that ... They never saw her again.

The wizard with Thumbelina remained on the ground to watch the ball quickly decrease in size.

Moral: We should learn to distinguish our desires from others.

The use of the tale also helped to reveal the **key conflict (between honesty and courtesy**). The politeness of the patient did not allow her to limit communication with unpleasant people. Thus, she was constantly under stress. And this restriction made a rash for her. The skin, as the boundary between the outside world and the human body, began to respond to rashes. The tale also helped the patient to see for herself what is really happening in her life. The ending of the story has a happy outcome and at the stage of ***Broadening of Goals*** the therapist drew the patient's attention to her internal resources and knowledge of how to solve the problem with the symptom.

In PPT, there are other original ways of working with a fairy tale. So, Werringloer [28] in his book "The Little Kite Flyer" suggested using the author's fairy tale in **pedagogical psychology** as an educational and pedagogical material for children and their parents in the Positum approach. As a metaphorical image of the **balance model**, which is one of the basic concepts in the PPT, the author created an image of a kite. This kite has the shape of a rhombus, actually a balance model, and the father explains to the young aeronaut the meaning of each sphere in a fairy-tale statement that the child understands.

> *Do you see this quadrangle in the form of a diamond? This form is the basis for a safe aircraft. It is designed for human proportions and is therefore extremely easy to control in the air. This diamond will also remind you of four directions: body, work, friendship and vision of the future. These four aspects are what life comes down to. If you want to lead a happy and fulfilling life as an aeronaut actor, then always remember this and try to keep the four aspects in balance. They will also help you succeed in competitions because they will provide you with strength, stability and peace of mind* (Werringloer [28]).

Thus, the PPT at the present stage of development continues to develop the ideas of Nossrat Peseschkian and proposes new forms of work with patients with the help of different types of stories.

Cinema Is like a Story. Working with Movies in a Group

All existing types of stories today can be seen and lived in films. Films can be short, comedic, with a psychological plot, romantic, contain mythology, etc. Very often, the patients independently during the session recall the plot from the cinema, compare themselves with someone from the famous movie characters, and identify themselves with them. We can say that the modern stage of work with stories, parables, fairy tales, anecdotes, and metaphors is also connected with film stories. Films are well remembered at the stage of situational encouragement and verbalization. Thus it includes not only a story but also an image, a complete picture.

In our work, we apply the cinema therapeutic method in practice. The work of a closed dynamic group, consisting of 8–10 people, usually takes place once a month for 6 h. After the greeting and sharing, the participants jointly watch a film selected by a therapist in advance, based on the group's objectives and psychodynamics. In each session, the group watches and discusses a new film story. The discussion of the film begins with the pronunciation of the phenomena—feelings, emotions, and bodily sensations that were discovered during the process of viewing, and then the therapist invites the group members to analyze the plot and think about how it responds to everyone's personal story [20].

Case Example: "A Monster calls"

At the **Observation stage,** the movie "A Monster calls" was watched (directed by Juan Antonio Bayona, 2016). The film is about a macro traumatic story that occurred in the life of the main character, Connor. Connor is a boy of 12 years

old, his mother fell ill with a deadly disease, and his father has long lived with a new family in another country. Connor has to grow up early, he feels a strong horror and fear of losing his mother. A magic tree comes to help the child—a huge monster of yew. This monster tells Connor several stories and thus prepares the boy for the inevitable loss.

At the **Inventory stage,** the group members exchanged their emotions, feelings, sensation, experiences about the plot itself, the characters, and special emotional inclusions in one or another episode of the film.

At the **stage of Situational Encouragement,** the group leader identified the boy's resources and mental abilities that helped the protagonist deal with the situation. The use of fantasy and the processing of trauma through dreams and imagination, helped the boy to live through a difficult situation of unbearable grief, and prepared him for life without a mother. His creative abilities gave him the strength to endure.

At the **stage of Verbalization,** the group members spontaneously began to recall and pronounce traumatic situations from their lives, how they lived the grief, shared how they overcame the pain, accepted the loss, what their own experience was, thereby revealing their greater involvement and identification with the world of the protagonist.

The Broadening of Goal stage was to integrate and assimilate the experience gained. The group members gave each other many warm words of gratitude and support for the opportunity to process macro-traumatic experiences in a safe space.

Thus, thanks to the group work with the cinema history, the participants were able to identify themselves with the main character, his life way, resources, and abilities to cope with difficult life challenges.

The Use of Humor and Anecdotes in Positive Psychotherapy

Generally, humor implies a kindly mocking attitude to something, an expression of the emotional perception of reality. Humor is also one of the ways to discharge—the transformation of negative feelings into the very opposite—into a source of laughter. The role of humor in this case comes down to the protection of the human I, because it allows you to maintain self-control, dignity, and self-control in exceptional (extreme) conditions. Such an understanding of humor in psychology has its origin in S. Freud, for whom humor was "a means of receiving pleasure, despite the agonizing affects that precede it" [7]. Humor suppresses the development of this affect, taking its place. Moreover, the pleasure of humor arises in these cases due to the unrealized development of the affect "it derives from the saving of affective expenses." In Freud's view, humor removes the contradictions between the controlling Ego and the subconscious mind. Freud said humor can be understood as the highest of protective functions [7]. The anthropologist Douglas, M. T. continues the Freudian tradition and argues that humor is an attack on control [6]. Modern psychological science also stands on the position that humor is a mature defense mechanism. Roman Ciesielski describes humor as a psychological defense in this way: "The expression of one's feelings in the form of irony or humor in order to distance oneself from one's experiences" [5].

Remmers [26] confirms: "The liberating laughter after the story unties the 'neurotic' knot in therapy, stops being trapped in an inner vicious circle. Humor is like salt in the soup of therapy, accelerating insight and relativizing fixed concepts. In depth psychology, humor is understood as the most mature defense mechanism to overcome resistance to change in therapy, which should have been understood as the ability to hold on to the same old things and resist change. Like proverbs, stories are one of the most effective means of overcoming resistance." But when in the psychoanalytic tradition humor is usually defined as a protective function, in Positive Psychotherapy Nossrat Peseschkian suggested using parables and stories not only to relieve the patient's tension, but also in a situation of limited time as an intervention. This function was played by the parable "A good example" told by Peseschkian to his patient, a frustrated father in a

briefcase from the book "The Merchant and the Parrot" [23]. Of course, intuition and professionalism are needed in order to subtly feel the possibility of therapeutic intervention in each particular session.

Anecdotes play a special role in PPT. Anecdote contains a brief story with an unexpected and ridiculous finale, thus facilitating further communication, having to contact. As a brief story, the anecdote carries an emotionally charged **metaphor**. First of all, this refers to well-known anecdotes, to phrases that have become sayings and can communicate information with the transfer of certain feelings and emotions, thereby finding common points for contact [29]. Short witty content also contributes to easy memorization, stress reduction, and is a demonstration of a person's **actual capacities**. Anecdotes help unite goals and change familiar ideas, make it easier to perceive the reality, and have a positive effect on emotional state. At the **stage of Situational Encouragement** as an alternative point of view, the therapist can use anecdotes to work with the patient as one of the most accessible and understandable ways to demonstrate a new pattern of behavior.

Anecdotes, as well as parables, have their own **morality**. It may be veiled at first glance. Through humor, a lesson learned from an anecdote is remembered better, takes the patient out of shame, awkwardness and anger and other heavy emotions, helps to overcome resistance. But the goal of humorous therapeutic stories is not so much moralizing as the opportunity to imagine an exemplary life situation and find yourself in it.

So, in the anecdote there is a resource that can be used at the stage of **Stimulation** and **Encouragement** during a consultation or psychotherapy, to discover **actual capacities** and their use; at the stage of **Verbalization**—anecdotes help to recycle complex feelings, to accept and discuss resistance, to change a position, etc.

Anecdotes due to their brevity, humor and metaphor can also be used for a **positive interpretation** of the situation.

Case Example: Humor

Moisha, I heard you divorced for the fourth time! Are all women so picky?

- No, only my mom

Anecdote, unambiguously hinting at the strongest maternal influence, and never occurred separation. And here the mother is the woman who destroys the life of her son, but there is no obvious criticism of her mother's address.

The anecdote becomes a **transcultural** phenomenon anecdote from the very beginning of its occurrence, because it has been told for a long time in a particular locality, it becomes part of the culture and, to some extent, is a concentrated content of information about a people, people of one region or one profession. Anecdotes, being easy to read and easy to remember, become a **model** and a **transcultural** way of communication. Due to common interests and problems, anecdotes from different nationalities in one locale are often identical, which can be a good incentive for the development of a common theme. Awareness of oneself and one's people connected with another culture is beneficial for intercultural communication [29]. Thus, the anecdote can also perform the **function of a social psychotherapist** [29].

In each culture and national community one can find a series of popular anecdotes emphasizing one or another peculiarity of the mentality and their developed actual abilities. These anecdotes are so recognizable that in a single **phrase—a metaphor**, it is clear what kind of story is being said. With this particular narrative, the anecdote appears as another feature of the stories, peculiar to this particular genre—the **Recognition Function** [29].

Through this **Recognition Function**, residents of one locality can, using a set of well-known metaphors, as an identifying mark and a sign of one community. The same function can be used in the contact between the patient and the therapist, which allows you to quickly establish a safe contact and therapeutic alliance.

Case Example: Anecdote

A *Frenchman, an American and a Ukrainian were told that t how much they would ride a horse so much land would be given to each.*

The Frenchman drove a kilometer, stopped the horse and said: "It is enough for me. Here I will build a house. There is a flower garden. And there I will have a rest on the sun loungers. And there is a vineyard."

The American traveled three kilometers and stopped the horse: "I have enough. My villa will be here. There's a lawn. There's a helicopter's playground. And even further, a golf court."

The Ukrainian jumped on his horse and drove ... Drove - drove the horse, drove – drove... He drove the horse, the horse fell. A Ukrainian on his feet and run! Ran, ran ... stumbled, fell. Already feels that he can no longer run. Crawling, crawling - everything hurts, it can no longer crawl. It takes off the cap from its head, throws it out of its last strength in front of itself and groans: "And there I will also plant cucumbers!"

A similar anecdote is found in Bulgarians, Moldavians, and some other nations living in southern Ukraine. Humorous and metaphorical statement "Also for cucumbers." Can change to "even for tomatoes" or "... and this is for me and pepper." These are uniting and recognizable metaphors for people who are able to desire and achieve more, to be ready for hard work (tillage), the ability to be hardy in achieving goals. Several **Secondary Actual Capacities** come to the fore, such as: diligence, frugality, conscientiousness, and **Primary Actual Capacities**: patience, hope, faith, confidence in ability, and determination.

Thus, the role of anecdotes in everyday life becomes significant in a positive and transcultural aspect and can be widely used in psychotherapeutic practice.

Case Example: Use of Humor

Dad-psychotherapist wakes up at night and goes to the fridge. He opens the door and sees his frozen son there.

– Dad, save me! Get me out of here!" The son asks.

"Stay with this," the father replies and closes the fridge door.

And this anecdote is often told by PPT trainers in groups of personal experience, where candidates for psychotherapists, examining themselves, meet with their complex experiences, resistance, and the need to be "here and now" in their "fridge" to give themselves time to living and rethinking. The ironic and humorous form of the anecdote smoothes the complexity of the process and underlines its necessity. The phrases "Stay with this," "Have you already come out of the fridge?" Denote a specific situation of professional growth, when space and time are needed before becoming a psychotherapist.

Nossrat Peseschkian in each of his works describes the use of stories in various life situations. He emphasizes: "If they are applied on time and in accordance with the recommendations, they can be the starting point for the therapeutic efforts of the doctor and contribute to changing the patient's life position and behavior. However, incorrect dosage, insincerity and the underlined moralizing by the therapist can cause harm." (Peseschkian [23]).

Kolenda-Sujecka [14] also suggests that satire is a special way of using humor in the therapeutic process. "Satire is a literary work that ridicules and stigmatizes human vices, customs, social relations, etc." One of the first to appreciate the therapeutic power of satire was N. Peseschkian, who used characters such as Hakim (doctor) and Nasreddin with great skill. Nowadays, according to Kolenda-Sujecka, the most frequently used form of satire in media reports is the satirical drawing, which is a kind of commentary on political events, customs, or social phenomena. Increasingly, satirical drawings also address psychological phenomena, internal or interpersonal conflicts, feelings, behavior, or character traits. Through the use of humorous elements and hyperbolization, satire reduces the emotional charge accompanying a topic, allowing us to see in it as if in a mirror and express that part of ourselves that is difficult to access in verbal communication [14].

Characteristics of Good Storytelling for Therapists

Teach me and I will forget. Show me and I remember. Involve me and I will understand. (Confucius)

Here is a short list of qualities of good storytelling and characteristics that therapists find helpful when sharing stories with their clients. It is by no means a comprehensive list and the right approach depends on the patient and the therapist [25].

- Observe the listener and adapt—if necessary—the storyline to the patient's situation.
- Narrate effectively. Avoid unimportant details and develop the core of the story. Take time to tell the story, using gestures, facial expressions, voice modulation and pauses; they provide emphasis for key parts of a story and enhance the client's ability to remember them.
- Discuss the story's likely applications to the patient's situation.
- Embed storytelling well in the session. There are many ways to do this. Frequently you won't need to explain the message and anyway, an explanation would detract from the patient's own reflections. Rather pause, let them associate with their own thoughts about the story. Possibly the listener will come up with something the therapist had not considered. You could also ask the patient, "How might the story continue?", "How would you have reacted in this situation?", "What does what you just felt/said remind you of?"
- Use a story to gather more implicit information about the patient's background. Questions that might be asked: "What were your favorite childhood stories?", "Which stories taught you values?", "With which heroes did you/do you identify?", "Which stories do you tell your loved ones?". Answers to these questions often yield more in-depth information about a patient than questions that ask directly about their background. It is also feasible to invite a patient to share stories they like in the next session (or to give other story-related homework).
- Use trance and mindfulness exercises together with a story (especially for longer stories) as research suggests their high efficacy in combination (c.f. e.g., [1, 9]).

Summary In Positive Psychotherapy, stories, anecdotes, and humor are deliberately used to enrich the therapeutic process. They act as mirrors, helping patients to reflect on their own narratives, and as models for illustrating pathways to conflict resolution. Stories enable patients to process personal experiences less confrontative and foster personal growth. Humor is employed as a means of relaxation and as a bridge to new perspectives, while anecdotes facilitate the exchange of cultural values and the overcoming of resistances. Good storytelling in Positive Psychotherapy is characterized by its ability to engage the listener, adapt to their situation, and convey the core message succinctly. Effective storytelling involves using gestures, facial expressions, and voice modulation to enhance memory retention and emphasize key points. By allowing patients to draw personal connections without overt explanation, stories enable a space for self-reflection and a deeper understanding of their own experiences.

Or with the words of Nossrat Peseschkian: "... along with extraordinarily entertaining, poetic and vivid presentations, they contain something unexpected, unforeseen. The usual course of thoughts and desires suddenly appeared in a completely different light. A different way of thinking, which seemed previously unusual, became close and clear to me. This change of position I consider the most important function of the stories." (Peseschkian [23]).

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Supervision in Positive Psychotherapy

37

Pavel Frolov

Introduction

Positive psychotherapy (PPT) was founded in 1968 as differential analysis. The first book on PPT was published in 1977. Since that time, PPT has developed not only as an academic, but more so as a clinical method, extracting knowledge primarily from medical practice, and only secondarily from the academic process. Therefore, supervision in PPT was oriented more toward the needs of professional psychotherapists in their work with the patients in hospitals rather than toward academic researchers. Even though formal training in PPT was established later, supervision has been used for decades, both by practitioners who have already completed their education, and by trainees in the process of education in psychotherapy.

There is no single definition of supervision, and probably one of the most comprehensive and detailed ones is Milne and Watkins' [11] definition based on the work of Bernard and Goodyear [2]:

> The formal provision, by approved supervisors, of a relationship-based education and training that is work-focused and which manages, supports, develops and evaluates the work of colleagues. It therefore differs from related activities, such as mentoring and therapy, by incorporating an evaluative component and by being obligatory. The main methods, that supervisors use, are corrective feedback on the supervisee's performance, teaching, and collaborative goal-setting. The objectives of supervision are "normative" (e.g., case management and quality control issues), "restorative" (e.g., encouraging emotional experiencing and processing, to aid coping and recovery), and "formative" (e.g., maintaining and facilitating the supervisees' competence, capability, and general effectiveness).

In their research, Simpson-Southward et al. [18] analyzed 52 different models of supervision, and despite the fact that all of them have used different approaches to forms and styles of supervision, all modern approaches are relatively united in their views of supervision as a professional activity.

Such a generalized view of supervision as a professional activity recognizes several of its inalienable functions, such as:

1. Becoming a competent therapist. According to Milne and Watkins [11]: "Perhaps the best-recognized function of supervision is to enable supervisees to become competent as psychotherapists."
2. Development of the therapist's abilities. This function addresses the practitioner's need for continued development in the course of daily work, and that of the supervisor to nurture the supervisee's ability to grow, emphasizing professional qualities, which should be prioritized.

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_37

3. Creating a professional identity, engaging the supervisee in the development of ethical approaches that enable the supervisee to fulfill professional expectations.
4. Providing the supervisee with the opportunity to develop certain competencies (to achieve certain qualifications), when the supervisor indicates which specific competencies are lacking and which need to be nurtured for him/her to become a professional.

All the above-mentioned functions of supervision, in essence, ensure implementation of the primary function: the provision of safe and effective therapy, i.e., clinical benefits. Although most experts agree on the importance of supervision in the professional development of psychotherapists, what is often overlooked is the primary goal of supervision: to promote safe and effective clinical practice, that is, when effective supervision improves therapeutic outcomes in patients.

According to the map of supervisor's competences [15], some common competencies of the supervisor include the ability to:

1. Teach practicing within ethical standards.
2. Adapt supervision to organizational and governance context.
3. Develop skills in dealing with differences while taking into account cultural considerations).
4. Establish and maintain a supervisory alliance.
5. Gauge the supervisee's level of competence.
6. Recognize and act on limitations in his/her own knowledge and experience.

Watkins [23] identifies six fundamental areas of professional competencies of a supervisor:

1. Knowledge and understanding of various models, methods, and interventions of supervision.
2. Knowledge and ability to address ethical and legal aspects.
3. Knowledge and skills in managing the relationship within the framework of the supervisory process.
4. Knowledge and skills in delivering judgment and evaluation.
5. Knowledge and skills in creating a therapy environment marked by openness to differences and diversity of approaches.
6. Ability to self-reflect and conduct an adequate self-assessment in the process of supervision.

Given the diversity and complementary nature of these competencies within the framework of supervision, there is never a question of prioritizing or choosing between them at any given moment. Generally, they are all important in each case, and all are present in each supervision session. Therefore, the task at hand is rather to apply all competencies in proportions appropriate to each individual session of supervision with each supervisee.

This proportion determines whether the supervisor is more supportive, succeeds in forming the supervisory alliance and developing supervisory relationships; or whether he/she focuses more on certain aspects of the therapeutic situation; or whether he/she confronts the supervisee regarding the ethical considerations of his action. Thus, the distribution of competencies may depend on many factors: the individual characteristics of a patient; the needs of the supervisee, his/her personal characteristics, and professional experience; the stage of the supervisory relationship; the stage of a particular supervision session, etc.

Positive supervision is based on the concept of a person as a creature consisting of capacities and inclined to develop, and at the same time possessing some advantages and limitations, based on the cultural context, which provides an understanding of the social norm. This is a cross-cultural analysis of the so-called "actual capacities" [21]. One of the important concepts lies in an immanent feature of actual capacities, which is their propensity to develop.

Thus, looking ahead, it can be said that supervision in PPT in a broad sense implies not only rational comprehension of the therapeutic situation by its participants but also the development of their own actual capacities, based on the needs of the supervisee and his/her patient within the therapeutic situation.

During the formation of professional supervision, its model and tasks, and the profession of supervisor itself, have been subjected to repeated revisions and development. However, the part that dealt with the relationship between the supervisee and the supervisor has always been understood to be crucial in this professional activity during the last few decades of research and development of supervision [1].

According to the theory of PPT, primary actual capacities, such as trust, acceptance (love), patience, hope, confidence, inner unity, etc., cannot be developed simply by thinking about them or by rational comprehension. These abilities can be developed only by means of example (modeling) in terms of real relations with a significant other. In this sense, the supervisee's lack of these primary capacities is often the main cause of complications or even the impossibility of successfully working as a therapist.

At the same time, secondary actual capacities, such as justice, diligence (achievement), accuracy, reliability, loyalty, punctuality, thriftiness, obedience, etc., being demonstrated by a supervisor, on the one hand reflecting the values and customary patterns of the supervisor him/herself, on the other hand forming certain situations of clarity, stability, and controllability of supervisory relationships, allow anxiety to be reduced, creating a secure environment of interaction.

These secondary capacities can be developed within the supervision rather than through direct training by the supervisor, encouraging their manifestation, both during supervision and in therapeutic work. Although, in fairness, the supervisor providing an example of politeness, openness, punctuality, thrift, justice, etc., can also be useful for the end result.

In this sense, supervisory relationships are the necessary environment, the space where these capacities are manifested by the supervisor in an appropriate measure and manner, when he trusts the supervisee, his opinion, impressions, conclusions; supports him in difficulties and confusion; encourages in his despair and inspires in helplessness; patiently endures the limitations of the supervisee and his inability to do something; draws the supervisee to his values and ethics; enables him/her to discover the meaning of what is happening in the therapeutic situation.

As the supervisee experiences such deep, sincere, and congruent interaction with the supervisor enables him/her to focus on his/her own ways to use the actual capacities, and use them in their relationship with the patient in the same way the supervisor used them in the supervisory alliance. Thus, it is a process of not only rational deepening of the patient's understanding of the current therapeutic situation but also the emotional development of the supervisee through supervision.

Previously, opinions were expressed that supervisees often simply imitate the behavior of their supervisor and just say what the supervisor wants to hear [3].

However, experience suggests that such a state of affairs might be possible only during the initial period of forming supervisory alliance and is a manifestation of the supervisee's need for support from the supervisor. In future, the skills and experience of relations are internalized and become the supervisee's own ones, available for future professional development.

Therefore, even if during the early stages of the supervisory relationship, it is just imitation through the use of the same words, expressions or intonations, then over time, the actual capacities behind these manifestations will develop, the simulated behaviors and internal values (concepts) are internalized, forming what was called the "internal supervisor" [20]. At this stage, the different aspects of the supervisor's behavior, attitudes, values being internalized, become an internal support for the more autonomous and independent work of the supervisee, developing his/her own therapeutic style.

Key Conflict in Supervision

The concept of the key conflict in Positive Psychotherapy may initially appear simplistic and even naive, as it describes a person's conscious or partially conscious choice between openness and politeness. This means that in any given situation of actual conflict, an individual

must choose between openly and honestly expressing their feelings (demonstrating the capability of "openness/honesty") or keeping their emotions to themselves (using the capability of "courtesy/politeness"). In practice, however, this issue is far from straightforward, as each of us creates our own unique ways of balancing and expressing both of these capacities simultaneously.

As mentioned earlier, the supervisor's role is quite complex, given the need to balance various supervisory functions: support, evaluation, and the education of the supervisee. For nearly all the supervisors I know, finding the balance between being honest and being polite towards the supervisee is a challenge to some degree. In a supervision setting, supervisors often find it easier to lean towards politeness and avoid giving honest evaluations of the supervisee's work.

There are several possible reasons for this. Partly, it may be related to the method of Positive Psychotherapy itself and its concept of personality, which views individuals in terms of their capacities and strengths. An essential part of being a professional in this field is the ability to recognize a client's resources, capacities, and areas for growth. Naturally, the supervisee's personality is viewed through the same lens. Since the bulk of supervisors' work involves their own clients in their role as psychotherapists, the familiar pattern of relationship-building with a client can easily transfer onto their interactions with the supervisee.

Additionally, giving compliments to a supervisee is far more comfortable than pointing out areas where they are struggling, which can elicit difficult emotions. Sometimes it seems that, as professionals, we ourselves may not be sufficiently equipped to handle confrontation, leading us to choose politeness and conceal some of our impressions rather than engage in honest discussions about the supervisee's challenges.

On the other hand, sometimes a supervisor's inability to provide open feedback can stem from a perceived lack of support skills. The supervisor may fear that they won't be able to support the supervisee through their frustration and therefore choose to withhold part of their observations. This behavior deprives the supervisee of crucial knowledge about their limitations and, as Nancy McWilliams [10] points out—who emphasizes the value of honesty in supervision—deprives them of models of straight talk.

In a study on effective and ineffective supervision, Ladany [8] frames this as a supervisor's ability "to deliver constructive criticism in the most positive manner, coupled with positive feedback."

Thus, although it seems always like "walking on thin ice," it remains a solvable challenge for the supervisor: finding the right form of politeness to be sensitive to the supervisee's emotions, while also being honest and direct enough to convey their impressions of the supervisee's work. If a supervisor is too blunt, their words may hurt the supervisee to the extent that they cannot grasp the feedback's content or benefit from it. Conversely, if the supervisor is overly polite, the supervisee may fail to recognize their weaknesses and limitations, which could hinder their professional development.

Five Stages of Positive Psychotherapy in Supervision

There is a specific five-step strategy of professional actions in PPT that is used in supervision [13]. PPT is considered a semi-structured method, but supervision, as an important tool for the professional development of a supervisee, is a well-structured process, with clearly defined stages and tasks facing the supervisor and supervisee at each of these stages.

Although the content of each of the successive stages may differ considerably from case to case, from supervisor to supervisor, however, the sequence of stages itself remains unchanged for all supervisors and each particular supervision, and looks like this:

- Stage 1 "Observation and distancing"
- Stage 2 "Making an inventory"
- Stage 3 "Situational encouragement"
- Stage 4 "Verbalization"
- Stage 5 "Broadening of goals"

Stage 1: "Observation and Distancing"

At the first stage, the supervisor provides space and time for the supervisee's spontaneous statement of the relative case, its formal aspects (duration, frequency), the content of the patient's difficulties, his experiences and the goals of psychotherapy, the supervisee's own experiences, in addition to the difficulties prompting him/her to bring this case to supervision. The supervisor during this stage encourages narrative, emotionally supporting the supervisee, observes his expression, experiences, builds hypotheses and conclusions, collects emerging questions, but refrains from bringing them up at this stage. At this stage, and at the next stage, the supervisor has a specific task stemming from the concept of personality and its development.

This task of the supervisor is to detect and identify certain aspects of the supervisee's behavior that seem to be valuable and useful in the therapeutic situation; successful interventions, interesting interpretations, in addition to actual capacities shown by the supervisee in the therapeutic situation.

The important skill for the supervisor is to see the supervisee's successes and capacities, and accurately verbalize them, which is critical in PPT and must be developed to a large extent. Its importance is separately explained in the description of stage 3.

At this stage, the supervisor establishes what is called a "supervisory alliance," the importance of which cannot be overestimated, given that one of the supervisory goals is to provide the experience of being unsuccessful, to acknowledge mistakes and limitations. Thus, it is important to empathize with the supervisee, to support him/her emotionally, and to encourage his/her efforts from the first stage of supervision.

As supervision, like psychotherapy, is not always a comfortable process, it is important that the supervisor and supervisee establish a good learning alliance [22]. A learning alliance implies that both supervisor and supervisee are allies with a common goal. Indeed, in various studies it has been suggested that the supervisory alliance might be "at the heart of effective supervision" [6].

Usually, the first stage is completed when the supervisee has exhausted information for spontaneous presentation, and the supervisor clarifies the difficulties of the supervisee in the case and expectations from supervision.

The most frequent requests in the framework of supervision are concerned with the diverse needs of the supervisee:

1. To understand the nature of the difficulties of the therapeutic case.
2. To better understand the patient, qualify him/her clinically (nature and level of disorders).
3. To understand the appropriateness of the therapeutic actions (interventions).
4. To see mistakes or wrong decisions and assess their consequences.
5. To understand the dynamics of relationships, causes, nature, and content of conflicts unfolding in a relationship with the patient.
6. To understand his/her own countertransference experiences that are causing difficulties in the case.
7. To discover unconscious aspects of attitudes and behavior toward the patient.
8. To express the feelings, including complex or "unacceptable" ones for the therapist, such as anger, fear, anxiety or panic, a feeling of helplessness or disappointment.
9. To get support in a difficult internal situation caused by the complexity of the therapeutic case.
10. To discover professional limitations and internal conflicts affecting efficiency in this particular case.
11. To see the professional strengths and skills in the current situation.
12. To identify external and internal resources of the patient and the therapeutic situation itself that he/she could rely on.
13. To understand the future prospects and possible tactics of strategy within the framework of the case.
14. To determine professional behavior in ethically challenging situations and potential legal complications, etc.

After clarifying the request, a verbal agreement is established regarding the purpose of the current session of supervision, which can also be described as entering into a supervisory contract.

Stage 2: "Doing the Inventory"

At this stage, the initiative shifts toward the supervisor, who by this time has sufficient information and is able to formulate questions. It is time to ask the questions to clarify all the aspects of the therapeutic case, based on the request of the supervisee.

Depending on the expectations and needs expressed by the supervisee, all components of the therapeutic and supervisory process can be investigated. In this sense, any part of the so-called seven-eyed model of supervision [5] can be the subject of research during supervision:

1. The patient's situation and content of the therapy session.
2. Technical tools and interventions used by the supervisee during the course of therapy.
3. Content and dynamics of the therapeutic relationship between the supervisee and his patient.
4. The therapist's processes (countertransference and other subjective experience).
5. Supervisory relationship between the supervisee and the supervisor.
6. The supervisor's processes (e.g., countertransference of the supervisor to the supervisee and the situation in therapy).
7. The environmental aspect (a wider transcultural context of the situation).

Whatever the request of the supervisee, factors directly related to the analysis of the patient and his situation are always dealt with, such as: the patient's balance model, identifying of actual conflict and its dynamics, its content in terms of actual capacities, areas and ways preferred by the patient to handle the conflicts, basic conflict, its content and connection with the actual situation, family concepts and their origins, and the other concepts of the differential analytical part of PPT.

This stage requires from the supervisor sufficient openness and directness to ask all the necessary questions and clarify all aspects of the therapeutic situation, to define contents, history, dynamics, and possibilities using the capacity to translate the emotional feelings of transference and countertransference into conflict contents, capacities, and relationship patterns [16].

The ability of the supervisor to adequately resolve his own "key conflict," that is, to maintain balance between directness and courtesy, the need to ask "tactless" questions while carefully considering the supervisee's capacity to endure confrontation, is crucial at this stage.

By the end of this stage, the supervisor should collect enough information and conceptualization to meet the request of the supervisee, whichever part of the complex supervision system it is concerned with.

Stage 3: "Situational Encouragement"

The stage of situational encouragement is unique because of its isolation in a separate part of supervision, as it relates to the important tasks facing the supervisor.

We have to admit that generally, the supervisee is not expected to feel comfortable during supervision. Rather, he is in a rather vulnerable position, as the supervision has traditionally been imposed on difficult therapeutic cases, causing the supervisee to have the most internal conflicts and difficulties that he/she may not be proud of. Having to ask for help from a supervisor does not make the supervisee feel successful and confident; on the contrary, he/she feels confused and second-guesses his or her actions, abilities, or further work tactics. This contributes to additional self-criticism.

Such a state of affairs never helps either rationally to analyze a complex therapeutic situation, or to encourage professional curiosity and the creative process; thus, one of the main tasks at this stage is to inspire the supervisee. The super-

visor should focus primarily on the supervisee's achievements and successes, and not on his/her mistakes and blunders.

At this stage, the supervisor devotes enough time to analyzing what in his/her opinion was done by the supervisee correctly and successfully. The supervisor honestly and sincerely shares what he noticed during two previous stages regarding the actual capacities, manifestations of the supervisee both in his/her work in relation to the patient and the therapeutic situation, and during the presentation of the case within the supervision session. The supervisor must be able to point out aspects of the supervisee–patient relationship that were useful, any successful interventions, capacities, and traits of the supervisee that have proved effective; skills and competencies that are sufficiently developed: anything that resulted in professional respect, approval, or even admiration from the supervisor. At this stage, it is especially noticeable that an experienced supervisor offers horizontal relationships instead of vertical, hierarchical ones, as this is what makes the supervisee more confident, allowing him/her to learn a respectful attitude toward him/herself and the patient.

It is very important that the supervisor has the skill of situational encouragement, because everything that will be pronounced must be part of objective reality, and it is very important to avoid baseless praise. The balanced opinion expressed by the supervisor should be the result of focusing on positive aspects during the previous two stages, the ability to argue their impressions of the strengths and behavior of the supervisee. This should not turn into merely an "emotional bribe" or flattery of the supervisee. At this stage, the supervisee has a substantial amount of information confirming what has been done correctly and successfully. The supervisor draws on the strengths of the situation, on the external resources of the therapeutic situation on which the supervisee can rely in his/her work, in addition to his/her internal competencies.

The result of this stage is a more level attitude to what is happening in therapy, a sense of relief, reduction of anxiety, guilt, and hopelessness in the supervisee, a sense of hope and inspiration regarding the therapeutic reality, and their own capacities and actions. Over time, by regularly participating in supervision, and observing such behavior in the supervisor, the supervisee learns to be more impartial toward him/herself, focus on his/her strengths, see his/her skills, build on them, readily acknowledge achievements and successes, thus developing a more conscious and holistic understanding and acceptance of himself as a psychotherapist.

Moreover, one has to admit that the difficulties of those who are supervised in a therapeutic situation are often associated with a deficit of primary actual abilities, such as hope, acceptance, trust, patience, etc. The part of the relationship exhibited by the supervisor toward the supervisee, especially at this stage, is absorbed and reproduced later on in the therapeutic situation in relation to patients. This is only possible if the supervisor does not simply suggest how the supervisee should behave in the therapeutic situation, but the supervisor manifests all these primary actual capacities in the living field of human relations with the supervisee within the framework of supervision.

To be fair, we have to admit that quite a few cultures do not develop the skill of attentiveness to success and healthy aspects, and this competence is sometimes difficult not only for supervisees but also for supervisors. Therefore, the development of this competence in supervisor training in PPT is given special attention.

Fundamental to this approach is the focus on the supervisor, demonstrating his actual capacities as they relate to various emotions and behaviors in the process of supervision. In this sense, the supervisor always creates the conditions for the supervisee to develop his/her current capacities and identifies the areas of development of the supervisee. To develop "actual capacities," the supervisor has to identify those already developed and those in need of developing; he/she has to cease opportunities and create conditions under which such development can occur. Successful development in this area is primarily conditioned on the relationship of support and acceptance,

respect, and recognition of merits, which is the primary objective at the stage of situational encouragement.

The self-disclosure of a supervisee is a rather common and accepted part of the effective supervisory relationship, which enables the supervisee to gain new knowledge within clinical supervision [19]. However, self-disclosure of the supervisor at this stage, when he/she shares his/her experience of difficulties, failures, or success, in addition to how he/she experiences a relationship with the supervisee, allows him/her to create warmer and more trusting relations within the supervisory alliance [7].

Different styles of relationships from their supervisees notwithstanding, all supervisors establish and develop a supervisory relationship.

One of the studies conducted by a group of researchers [4] found that the work of therapists was evaluated differently by their supervisors and by independent judges. All supervisors in three modalities, chosen by the researchers, rated their supervisee higher than independent judges. This is interesting, as the explanations ranged from the supervisors' excessive loyalty to their supervisees to their greater professional immersion in the context of therapeutic situations and their understanding of the situation of supervisors. However, perhaps one of the probable reasons is that supervisors could see the dynamics that distinguished the current work of the supervisee from his previous work, and could see the prospects for its development, thus referring to the supervisee, as though he or she had already become more experienced. There is a possibility that this is an unconscious mechanism of inspiration for supervisees, when their supervisors assess them in terms of development, admitting advances for success, which they will only achieve in the future. Perhaps this is precisely what is really needed to inspire the supervisees, to revive their optimism, allowing them to mobilize their strength to work and develop consistently. In this sense, one of the important functions of the supervisor is to see potential. Supervisors should take pains to discern it as part of their routine supervisory duties.

When the participants in supervision understand what strengths and resources of both the patient and the supervisee are, in addition to assessing the therapeutic situation in its entirety, only then do they proceed to the next stage.

Stage 4: "Verbalization"

This is the stage where a more comprehensive discussion of what is happening during therapy takes place. The supervisor shares his/her observations, hypotheses, and conclusions based on the request of the supervisee and his own vision of the situation. The ability of the supervisee to confront with the experience of his own mistakes and limitations becomes emotionally accessible because of the well-conducted previous stage, when anxiety and a sense of insecurity are reduced.

At this stage, a significant role is assigned to the supervisor, who, during the previous stages had the opportunity to observe what was happening in the therapeutic situation, and now can express his observations. The supervisor must not only conceptualize but also clearly verbalize his conclusions and observations. The supervisor has to be open enough so that, avoiding any criticism, he/she can nevertheless tell the supervisee what was done in vain or was lost.

At the stage of verbalization, the supervisor has the opportunity to share his countertransference experiences during supervision, which is very useful for illustrating and explaining what is happening in the therapeutic situation with the patient. In this sense, the generally accepted concept of parallel processes, when it is believed that dynamics in a supervisory alliance often mirror dynamics in a therapeutic relationship, is very useful.

The supervisor is expected not only to share his vision of the situation and difficulties of the supervisee but also to provide the supervisee with space to actively participate in creating new concepts and drawing conclusions from the supervisory work. Very often, if the work is done correctly, the supervisee manages to see what he/

she had not seen before. Therefore, at this stage, there is usually joint work, when the supervisor and the supervisee together create a more holistic and comprehensive vision of the therapeutic situation that has become the subject of the supervisory research.

By the end of this stage, the interest of the supervisee regarding the causes and nature of the difficulties presented for supervision is generally addressed, but the question of strategy and tactical actions that are needed in the future remains. This task constitutes the final stage of supervision.

Stage 5: "Broadening of Goals"

This is the stage when it becomes clearer what is happening in the therapeutic situation, and the supervisor and the supervisee develop a plan for further therapeutic actions, or at least outline the vector of further development of the therapeutic situation.

In addition to meeting the needs of the supervisee, it is important for the supervisor in his/her feedback to expand on ideas about what is happening not only in the area of interest stated by the supervisee but also in those areas that could escape the supervisee's view. The ability to see areas of development in the presented therapeutic situation, and in the supervisee himself, as a professional, is one of the important skills of the supervisor in PPT [13].

As the goals and objectives of supervision differ from the goals of therapy, those findings that relate to personal material of the psychotherapist arising during supervision and affecting professional behavior are usually indicated, but not worked through. They are recommended for research outside of supervision within the framework of personal therapy.

Transcultural Approach in Supervision

One of the important competencies of the supervisor is the transcultural approach, the primary objectives of which are understanding and accepting similarities and differences between the cultures of all participants in supervision: the supervisor, the supervisee, and the patient.

Despite the idea that cultural sensitivity is a competence that requires a person to be willing to address issues of race and ethnicity [9], in PPT the transcultural approach can be considered in a wider context as the capability to see the unique individuality and resources of patients and, in the case of supervision, the supervisee, within their specific social and cultural environment in three dimensions:

1. Comprehensive dimension implies similarities and differences in the widest context at the level of different continents, countries, and states, and not just national cultures. Accordingly, at this level, differences will encompass the largest spectrum of human life, relationships, and traditions.
2. Transcultural approach in a broad sense implies similarities and differences at the level of one country, but different ethnic groups (for example, despite a single citizenship, currency, constitution, and laws, Russia is populated by representatives of 194 separate ethnic groups), and each of them may be a carrier of its own—sometimes very different and even opposing—habits and traditions.
3. In a narrower sense, the transcultural approach allows similarities and differences to be found, even if individuals belong to the same culture, speak the same language, were brought up in the same city, or even grew up in the same apartment building. In this case, significant differences can be found in ideas about how to build relationships in the family, what values are fundamental to life, what is the value of an individual, etc., because these aspects vary considerably even in the same society from one family to another. Therefore, in this case, the carrier of similarities and differences is the parent family, as the narrator and carrier of family culture.

In this context, the transcultural approach implies the ability of the supervisor to proceed not based on their own cultural values and experi-

ence, but based on values and traditions of the culture of the supervisee and the culture in which he/she operates.

This should not be seen as a supervisor's denouncing of his/her own culture, which should be well understood and accepted, but it should be understood and experienced as his/her culture, rather than one that is superior just because he/she was brought up within this culture.

This does not mean giving up one's own culture, its values, and traditions. On the contrary, it has to be well understood, supported, and developed by the supervisor.

Here, the supervisor is required to understand his/her culture, precisely as being relatively valuable, with the recognition of the same value of the culture of the supervisee and his/her patient. The supervisor's ability to move away from the values of his/her own culture for the sake of understanding and supporting the supervisee or patient is one of the important competences, in addition to his ability to estimate the limit of difference that can be tolerated and accepted without deceiving himself and the supervisee. The transcultural approach implies the ability to get out of their usual cultural point of view to understand and share the cultural point of view of another, in this case, the supervisor or his/her patient.

Vignette

During supervision, the supervisor talks about the difficulties in family therapy of a married couple (Japanese and Russian), where, during the study of family conflict, it turned out that the trigger for a serious trial, including family therapy, was the discovery that the Japanese husband, with enviable regularity for many months, had been attending the so-called "*pinsaro*" (pink salon, blow-job bar), while engaging in sex with his wife quite infrequently. For the supervisee, the main difficulty in dealing with the family situation was the fact that neither spouse particularly focused on this circumstance, regarding it as an annoying misunderstanding. Their major difficulty in marriage was the lack of time spent together and attention given to one another. In countertransference, the supervisee had many experiences, including aggression, associated with the complete rejection of such behavior of her husband in marriage and an inability to imagine that this could really hurt partners so little. The supervisor rather regarded such lack of interest and reactions at this moment as the result of psychological defense mechanisms on the part of both partners. Anger and intense aggression made it difficult to focus on other aspects of the conflict and the family situation as a whole. There was a suspicion that patients either demonstrated resistance or intentionally misled the therapist, which seriously impeded the development of a therapeutic alliance.

In the course of supervision, the supervisee was given the task of studying the traditions of sexuality in Japanese family culture in general from a transcultural point of view, and further, the place reserved for the *pinsaro* in particular.

After conducting this research, the supervisee learned that this behavior is not only legal, but that it is only occasionally frowned upon under certain conditions, i.e., it is considered conditionally undesirable, and therefore, easily dismissed by all participants in a family conflict. Thus, the supervisee had to expand his/her ideas about the conditional norm in sexual and family behavior, which was fundamentally different from his/her own culture. In the end, he/she managed to separate the traditions of family culture (in full disclosure, it should be noted, after several sessions of personal therapy) so that he/she was able to focus on other aspects of family relationships presented by the family, and to help this couple.

To become more transculturally educated, the supervisor needs to study his/her own limitations, expand his/her transcultural experience, and resist the tendency to narrow down the type of preferred patients or the nature of their difficulties, limiting them only to Christians, women who have suffered violence, representatives of only one ethnic group, race or profession, etc. On the one hand, the higher the level of transcultural differences, the more difficult to understand, accept, and share the traditions of another person, the more emotional reactions they cause in a spe-

cialist. On the other hand, the more transcultural experience working with professionals from diverse cultural backgrounds the supervisor has, the easier it is for him/her to accept the relative value of his/her own cultural traditions and, therefore, to accept diversity in its many dimensions.

That is why, in the spirit of constant apprenticeship, curiosity, and openness, the supervisor should be able to interest and captivate the supervisee with other cultures and traditions, regardless of their transcultural differences. As a result, the more the supervisee studies the various traits leading him/her to love another culture, the more he/she learns to understand and respect its traditions and genuinely accept its differences from his/her own culture and traditions. Thus, a transculturally educated supervisor is one who has experience dealing with representatives of different cultures, who recognizes their equality and value, and knows how to deal with cultural similarities and differences.

The Use of Metaphors and Humor in Supervision

As the founder of positive psychotherapy Nossrat Peseschkian was Persian by origin and in the culture in which he was brought up, one of the therapeutic elements of the method is the use of metaphors, which were initially Persian parables. Having reached the age of 21, moving from Eastern to Western culture, finding the use of primarily rational thinking, and ignoring the power of imagination and fantasy, Peseschkian conceptually developed the idea of using metaphors in psychotherapy, which are now actively used in supervision. Over time, the list of therapeutic parables increased, the functions of metaphors were described in more detail [12–14], and the methods of metaphorical utterance became more diverse.

The main idea of using metaphors is to move away from habitual rational thinking to activate the supervisee's imagination, give him/her the opportunity to go beyond the usual view of what is happening, evoke more emotions, activate creative thinking, change and expand perspectives. All this can be very useful in the case of supervision, when the supervisee feels confused, frustrated, and helpless. The use of metaphors provides an expanded view of the therapeutic situation, the behavior of the patient or therapist, can help to see what is happening under a different, sometimes unexpected for the supervisee, angle, provides alternative ways of relating to a behavior in the therapeutic situation, reflects certain aspects of the supervisee's behavior that elude him, etc. The beauty of using metaphors in supervision also lies in the fact that, owing to their humor and paradox, they strip complex situations of perceived drama and hopelessness, the exaggeration of finality of the ending. It removes the supervisee's perception of his difficulties, helplessness, disappointment, etc., as being unique, and brings faith in his ability to resolve the issue.

The supervisor can use as metaphors not only parables, whether oriental, Buddhist or others but also experiences from his/her own life, his/her own experience as a therapist, cases of other supervisions, episodes from movies, TV shows, books, anecdotes, etc. It is even better if the supervisee can find his/her own metaphors applicable to the therapeutic situation or its various aspects. As one of the functions of metaphors, because of their emotional charge, is the storage of personal experience, it is then easier for the supervisee and the supervisor, in the event of continued collaboration, to access previous findings and conclusions, containing them in one or other metaphoric form.

In addition, the experience of supervision, gradually filled with its own metaphors, which have a very specific and personal meaning, known only to the two participants in the supervision, becomes the product of their interaction and helps to create that unique alliance that is necessary for effective supervision. As with all the tools of PPT, the supervisor should have not only the ability to design useful metaphors but also to use them in moderation, based on their adequacy for a particular therapeutic case, the nature of the experience of the supervisee, the relationship stage, etc. However, to paraphrase

Peseschkian's words about the importance and usefulness of humor in a therapy session, I would say: a supervisory session where a supervisee has not laughed at least once is a session wasted.

Group Supervision

Since the supervisory process is well structured, it lends itself well to group supervision as it provides practical benefits and a didactic effect. Among the many formats of group supervision in PPT, the most common one focuses on one case during a session with one supervisee, and all the rest of the group being participants, including the supervisor, who help the supervisee to deal with difficulties of the case. In a sense, the supervisor makes the other participants his co-supervisors, enabling the supervisee's view on the case to be enriched with other perspectives and the additional views of all the participants of the group.

In group supervision, the tasks and sequence of actions of all participants are very similar. However, the supervisor must be mindful that in the context of group work, where the supervisee is the object of observation and discussion, the supervisee is more vulnerable to criticism and judgment, primarily from the most influential participant—the supervisor—in addition to the other members of the group. They do not simply observe the supervisor in action, but also actively participate, responding to the encouragement of the supervisor to emulate his actions. Each stage, strictly outlined, has its own clear objectives. The overall objective of the supervisor and the group is to create conditions for the supervisee to feel so comfortable that he/she can be open to both emotional and intellectual experiences, without the need to defend him/herself, which results in getting the most out of the supervisory process. In fact, the characteristic sign of group supervisions going well is when participants come again and again and try to get their case selected for group supervision. The group supervisor must be attentive to his/her countertransference reactions and derived behavior, because, based on many years of observations, it is the behavior of the supervisor, not his/her words and didactic introductions, that determine what other participants learn. The more supportive the supervisor, the more the group supports the supervisee, and vice versa: the more critical the supervisor, the more likely the other participants are to focus on the mistakes and oversights of the supervisee and express criticism.

Most modern research in the field of supervision end with the words that we do not yet have sufficient evidence of the effectiveness of supervision on the patient's outcome [22, 24]. In this regard, we have to use this format of professional activity so far, relying only on empirical experience and our own, albeit subjective, assessment of its value, just as we continue to participate in supervision and develop it after its effectiveness has been proven scientifically.

To concur with Arno Remmers [17] about the identity of the positive psychotherapist, I could repeat his words about being a positive supervisor:

> I see myself as a learning human being in interaction with my supervisee, and my duty as a positive supervisor today is to prepare myself and my supervisee for the world of tomorrow by using a positive resource-oriented approach to resolve the conflicts we face and will face tomorrow.

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38 Spirituality and Religion in Positive Psychology and Positive Psychiatry

Karuna Poddar, Mark Famador, Shridhar Sharma, and Rama Rao Gogineni

Introduction

This chapter's main objective is to clarify the concept that religion and spirituality are the basis of positive psychology. The second objective is to show that religion and spirituality are not two mutually exclusive terms. Religion and spirituality have a positive correlation to psychological well-being. However, there has been much confusion over their operational definitions, concepts, and dimensions. Many associate spirituality as connected to religion. Vedic philosophy has identified spirituality to be a pre-religious concept [34]. Religion or religiosity includes more organized beliefs, practices, and systems that most often relate to the worship of an omnipotent force or figure. Hinduism, from what it is known today, Christianity, Islam, Jainism, Sikhism, and the other religions in the world, brings a community of people together based on their cultural views, prophetic beliefs, and morals, thus inculcating spiritual meaning of their faith-based system. Religion is about helping individuals find meaning in life, how to deal with life, and live in relation to transcendence. It helps to promote both inner and outer peace, harmony, and morality in both individuals and humankind. Spirituality on the other hand is often understood as more an individual practice and way of proceeding in life rather than an institutional affiliation. It often operates at a higher level. It is concerned with the soul and is directed toward the pursuit of personal meanings and reflects positive emotions; while positive psychology is that part of psychology that takes on the task of deciphering the mechanisms and the opportunity of nourishing positivity in life. How this has evolved in human history is both an interesting and a fascinating journey [1].

There have been philosophers who contributed to the idea of an "ultimate reality" as a unique and *the* central idea in both religion and spirituality. Swami Vivekananda's quotes (1863–1902) "Great is the tenacity with which man clings to the senses. Yet, however substantial he may think the external world in which he lives and moves, there comes a time in the lives of individuals and of races when, involuntarily, they ask, "Is this real?"…Religion begins with this question and ends with its answer." [42]. The

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_38

French intellectual, philosopher, and playwright Gabriel Marcel (1889–1973), a noted opponent of atheistic existential philosophy (the latter approach recognized for its view that life's experiences and interactions are meaningless), struggled with the concept of God. From the beginning of his career, Marcel's writings on religious belief are one of his most profound contributions to philosophy. His main interest has been the interpretation of religious experience, that is, of the "relation between man and ultimate reality" (Internet Encyclopedia of Philosophy). In this paper, the concepts of positive psychology will be viewed through the lens of various religions in the world (including some additions) will be explored. In the context of the Christian religion, the life of a medieval saint, Ignatius of Loyola will be used as a way to understand Christian spirituality.

The paper will describe surveys of religion and spirituality in our society, definitions of spirituality and religion, positive psychology, and positive psychiatry. The paper then describes the connections of these surveys and definitions from various philosophical, religious, and theological perspectives. Finally, the write-up describes *Ignatian spirituality* to highlight the subject of the paper in detail before making the concluding remarks.

Until the early nineteenth century, psychiatry and religion were closely connected. Religious institutions were responsible for the care of the mentally ill. A major change occurred when Charcot and his pupil Freud associated religion with hysteria and neurosis. Albert Ellis in 1980 wrote that there was an irrefutable causal relationship between religion and emotional and mental illness. At its most extreme, all religious experience has been labeled as psychosis. All these created a divide between religion and mental health care, which has continued until recently. But in 1994, "religious or spiritual problems" were introduced in *DSM-IV* as a new diagnostic category that invited professionals to respect the patient's beliefs and rituals.

Definitions and Concepts

Religion and spirituality have a positive correlation to psychological well-being. However, there has been a great deal of confusion over their operational definitions, concepts, and dimensions. Spiritualism ideally follows the concept of non-duality, also known as Adhvaita, a term commonly found in Vedic philosophy [40]. Spirituality is known to help an individual raise their level of consciousness and merge with the higher consciousness. Religion is known to follow the concept of duality, also known as Dhvaita. Religion is a concept easier for humans to understand, practice, and follow, with the ultimate goal of merging with higher consciousness through structured methodologies using duality-based beliefs, rituals, and practices. It's often done in a community setting with a focus on a relationship with *the transcendent* [30, 33].

Religion was adapted from the Latin term "religio," a term roughly meaning "scrupulousness" or "conscientiousness," "devotedness," or "felt obligation." Over the period of years, it became associated with the term "our way of worship," thus creating different groups identifying with a supreme deity, who is worshiped, thus cultivating virtue along with the sense of one requiring punishment or reward in their life and seeking repentance for wrong doing (Edward Herbert 1583–1648). Religion eventually got many definitions, either substantively or functionally. Thus, the original concept of scrupulous devotion was reorganized to refer to a type of social practice [30]. It is often found to help promote both inner and outer peace, harmony and morality in both individuals and humankind.

Archaeological finds indicate to us that spirituality was practiced by prehistoric humans (e.g., pagan practices), but much of the evidence is continually debated today. Spirituality has been known to humankind way earlier than structured religionism. Spirituality operates at a higher level. It is concerned with soul and spirit and is directed toward the pursuit of personal meanings

and reflects positive emotions, while positive psychology is that part of psychology that takes on the task of deciphering the mechanisms and the opportunity of nourishing positivity in life. How this has evolved in human history is both an interesting and a fascinating journey [1]. Spirituality usually refers to a dimension of human experience related to the transcendent, to the sacred, or to *an ultimate reality*. It is closely related to values, meaning, and purpose in life. Spirituality is known to enhance people's well-being so that they can respond to the well-being of others. It enables one to *being more* than oneself, and expresses the pursuit of responsible engagement [31]. Spirituality may develop individually or in communities and traditions.

Religion is often seen as the institutional aspect of spirituality, usually defined more in terms of systems of beliefs and practices related to the sacred or divine, as held by a community or social group (Koenig H, King D, and Carson VB). The act of believing in an "ultimate reality" appears to be a unique and central idea in both religion and spirituality. The French intellectual, philosopher, and playwright Gabriel Marcel (1889–1973) is a noted opponent of atheistic existential philosophy (the latter approach recognized for its view that life's experiences and interactions are meaningless). Marcel's philosophical writings on religious belief are one of his most profound contributions to philosophy. Marcel's main interest has been the interpretation of religious experience, that is, of the "relation between man and ultimate reality" (Internet Encyclopedia of Philosophy). Regardless of precise definitions, spirituality and religion are concerned with core beliefs, values, and experiences of human beings. It is my hope that in the discussion of the concept of positive psychology in the context of the life of a medieval saint, Ignatius of Loyola, these concepts and dimensions of religion and spirituality will become more clearly understood.

Positive psychology is based on a particular model of the individual that was developed in the popular culture of the United States from the end of the eighteenth century. This model, which we have called "positive" individualism, started with Emerson's transcendentalism which defended against puritanism (beliefs and behaviors based on strict moral principles). In Emmerson's transcendentalism, the individual was promoted as part of a divine essence, capable of self-command, self-exploration, and endless self-development. Current positive psychology moves away from the most striking metaphysical aspects of "positive" individualism, but maintains its historical, asocial, and subjectivist conception of the individual. Religion and spirituality have a positive correlation to psychological well-being [1]. For our purposes, well-being is defined here as an optimal experience of life.

Positive psychology embraces the whole person, one's strengths and weaknesses, as the cliché goes: "Warts and all." In a sense, it is living life to the fullest to the measure that one can. In the words of Christopher Peterson (2009), positive psychology is the scientific psychology of what makes life most worth living [3]. Viktor Frankl, founder of logotherapy, a form of existential analysis, tried to grapple with the meaning of life amidst the uncertainty of surviving through the concentration camps during the holocaust. He tried to share his own struggle to find meaning, fulfillment, and happiness in the face of suffering in his best-selling book, *Man's Search for Meaning* [8]. Philosophers from ancient Greek to modern ones have grappled with the basic question: "Who am I?" Socrates espoused: "The unexamined life is not worth living." Aristotle, philosopher and pupil of Plato, emphasized the importance of developing excellence or virtue of character as a goal to be achieved. The highest aims are that of living well and *eudaimonia* (a Greek word often translated as well-being), happiness or "human flourishing," or doing well in this life. Eudaimonia here is defined as "an activity of the soul in accordance with *arete*." *Arete* ("excellence" in Greek) here is defined as a distinct wisdom or knowledge achieved through

repetition, doing, or practice. Virtue here is moral virtue, or excellence of character, the disposition (Greek *hexis*) to act excellently, which a person develops partly as a result of upbringing and partly as a result of a habit of action [8].

In short, positive psychology seeks to make people happy by making life meaningful. Adopting the thought process of the ancient Greek philosophers, a meaningful life is characterized as living a virtuous life. More so, this virtuous life is partly a result of one's developmental experiences starting from family onward and partly (I would emphasize) a result of constant repetition and practice. Positive psychology is an "inner discipline," in search of realizing the truth about the self and the full potential of the human being. The goal is to find one's true, authentic self through a special kind of consciousness, meditative, or prayerful state. It is an activity not only of the brain but more of the mind, where there is an intuitive kind of knowledge born out of successive trials and errors where one discovers *one's disordered attachments* in order to freely pursue the path in life that will lead to fullness and happiness [2].

Furthermore, positive psychology is a dynamic process that must lead to further self-realization and service to others. It is a knowing that leads to being (K. Ramakrishna Rao) [9]. It is not monolithic but universal in that it has the potential to inspire whole cultures to achieve their better selves. The idea of "the better self" is not relative but can be generalized for all human beings. In order to nurture individual talent and make life more fulfilling, positive psychology focuses on three areas of human experience (as described by Seligman 2000). These three areas will help define the scope and orientation of a positive psychology perspective.

At the subjective level, positive psychology looks at positive subjective states or positive emotions such as happiness, joy, satisfaction with life, relaxation, love, intimacy, and contentment.

At the individual level, positive psychology focuses on a study of individual traits or the more enduring and persistent behavior patterns seen in people over time. This study might include individual traits such as courage, persistence, honesty, or wisdom. That is, positive psychology includes the study of positive behaviors and traits that historically have been used to define "character strengths" or virtues.

Last, at the group or societal level, positive psychology focuses on the development, creation, and maintenance of positive institutions. In this area, positive psychology addresses issues such as the development of civic virtues, the creation of healthy families and communities, and the study of healthy work environments and positive communities. Martin Seligman and Mihaly Csikszentmihalyi (2000) described positive psychology as "psychology of positive human functions, which helps to build thriving individuals and communities" [9]. Another concept extensively researched in positive psychology is "flow." Csikszentmhilayi (1990) has written extensively on "flow." Flow is the "state in which people are so involved in an activity that nothing else seems to matter; the experience is so enjoyable that people do it even at great cost, for the sheer sake of doing it" [9]. Csikszentmihalyi derived the concept of flow and its characteristics by studying a "few hundred 'experts,' – artists, athletes, musicians, chess masters, and surgeons." Later, his team of researchers and "colleagues around the world" interviewed thousands from different walks of life belonging to different cultures and age groups. They also tried to measure the quality of subjective human experience, the so-called subjective well-being.

Some Positive Statistics and Philosophical Explanations

Plato said that man is not only conscious, but he is "conscious of his consciousness." Scientists believe consciousness and thought are entirely physical products of our brain and are created by the electrical and chemical changes in the neurons in our brains. While philosophers who believe in spirituality claim that spirituality is concerned with the soul and it is directed for the pursuit of "personal meaning." A recent article from Harvard, identified that spiritual practices, with advanced meditation, alter consciousness

and individuals' sense of self (https://mghpsych-news.org/matthew-sacchet-advanced-meditation-alters-consciousness-and-sense-of-self/). It has been suggested that there is a difference between spirituality and religion. There exists however, an intimate relationship between spirituality, religion, and mental health. Science gets us physical comforts; spirituality brings us mental peace and raises our consciousness. Positive values, attitudes, beliefs, and strengths acquired through spiritual practices contribute to the sense of bliss. But spirituality is not just about following our bliss alone. Spirituality has a deep psychobiological basis and a reality that needs to be understood. The pleasure we derive is through our senses and is basically physical. While happiness, which is a positive emotion, is psychological. "Bliss" which is higher than "Pleasure" and "Happiness," is spiritual. This hierarchy of positive emotions will help us to clarify the concept of spirituality and its proximity with mental health [1].

Various spiritual traditions indicate that the nature of mind, consciousness, and reality as well as the meaning of life can be apprehended through an intuitive, unifying, and experiential form of knowing [1, 8]. A scientific frame of reference must address the evidence for that. Such a framework would greatly stimulate the scientific investigation of the neural, physiological, psychological, and social conditions favoring the occurrence of mystic experiences as well as the effects of such spiritual practices on health, psychological well-being, and social functioning. There is a newer trend in human evolution toward spiritualization of consciousness. The proposed new scientific frame of reference may accelerate our understanding of this process of spiritualization and significantly contribute to the emergence of a planetary type of consciousness. This type of consciousness is essential if humanity is to successfully solve the global crises that confront it and wisely create a future that benefits all humans and all forms of life on Earth.

At the beginning of May 2005, Gallup asked Americans to rate how important religion is in their lives. Of those surveyed, 55% rated religion as very important, and 28% rated religion as important. Only 16% stated that religion was not important at all. (These statistics are from Gallup.com.) Peterson and Seligman (2004) observed that spirituality is universal: "Although the specific content of spiritual beliefs varies, all cultures have a concept of an ultimate, transcendent, sacred, and divine force" (p. 601). If a belief in the transcendent is so much a part of the human experience, isn't it curious that research on spirituality and religion is so under-represented in the field? One explanation may be that the concept of spirituality doesn't fit neatly into our current research molds. ("Can you please rate on this 7-point scale how much the Spirit moved you today?") Even mapping the conceptual distinctions between what we refer to as "religion" and what we refer to as "spirituality" can be difficult. In their chapter on spirituality in the *Handbook of Positive Psychology*, Pargament and Mahoney (2002) make the distinction as follows:

> We prefer to use the term religion in its classic sense as a broad individual and institutional domain that serves a variety of purposes, secular as well as sacred. Spirituality represents the key and unique function of religion. In this chapter, spirituality is defined as a search for the sacred.....People can take a virtually limitless number of pathways in their attempts to discover and conserve the sacred.....Pathways involve systems of belief that include those of traditional organized religions (e.g. Protestant, Roman Catholic, Jewish, Hindu, Buddhist, Muslim), newer spirituality movements (e.g. feminist, goddess, ecological, spiritualities) and more individualized worldviews.

The Pew Research Forum on Religion and Public Life (2008) [24] had found the following:

1. More than nine in ten Americans (92%) believe in the existence of God or a universal spirit.
2. Sixty-three percent of American women and 44% of American men say that religion is very important to their lives.
3. Americans are nearly unanimous in saying they believe in God (92%), and large majorities believe in life after death (74%) and believe that scripture is the word of God (63%).

4. Most Americans (54%) say they attend religious services fairly regularly (at least once or twice per month), with about four in ten (39%) saying they attend worship services every week.
5. Americans also engage in a wide variety of private devotional activities. Nearly six in ten (58%), for instance, say they pray every day.
6. People who are not affiliated with a particular religious tradition do not necessarily lack religious beliefs or practices. In fact, a large portion (41%) of the unaffiliated population says religion is at least somewhat important in their lives, seven in ten say they believe in God, and more than a quarter (27%) say they attend religious services at least a few times a year (Pew Research).

The latest Pews research (2023) on Spirituality and Religion Amongst Americans found that, out of 10 US adults, 7 described themselves as spiritual in some way with 22% stating themselves to be spiritual, but not religious. In recent decades, it has been found that Americans have become less likely to identify with an organized religion. [24]

The survey finds that:

- 83% of all US adults believe people have a soul or spirit in addition to their physical body.
- 81% say there is something spiritual beyond the natural world, even if we cannot see it.
- 74% say there are some things that science cannot possibly explain.
- 45% say they have had a sudden feeling of connection with something from beyond this world.
- 38% say they have had a strong feeling that someone who has passed away was communicating with them from beyond this world.
- 30% say they have personally encountered a spirit or unseen spiritual force.

Overall, 70% of US adults can be considered "spiritual" in some way, because they think of themselves as spiritual people or say spirituality is very important in their lives.

Religious Orientation or Style of Coping

Gordon Allport, the famous personality theorist, made the distinction between intrinsic and extrinsic religious orientation. In other words, an extrinsically oriented person seeks out religion because it provides comfort and security; however, he or she would also be motivated by guilt or external sources of pressure (family, social pressure, etc.). In contrast, an intrinsically oriented person is motivated more by faith and a search for meaning and purpose in life. Some evidence suggests that individuals with an intrinsic orientation are better able to cope with stressful life events since this orientation leads them to find meaning in what has happened. Other researchers highlight how differences in one's religious/spiritual problem-solving style can affect our ability to cope with adversity. Four styles have been identified [25]:

1. A collaborative style—Individuals with this style see themselves as working with God to deal with the problem at hand.
2. A deferring style—Individuals with this style are more passive. They wait for God to handle the situation.
3. A self-directing style—Individuals with this style are calling the shots. Though they may believe in a higher power, they rely on themselves to solve/handle any problems.
4. A surrendering style—Individuals make a conscious decision to relinquish those aspects of the situation that are truly beyond their control.

The collaborative style seems to be adaptive in a wide range of situations in that individuals tend to feel empowered (with God on their side) and motivated to do what they can to improve the situation. The self-directing style is also generally effective, largely because people tend to fare better when they perceive a situation as controllable. The noteworthy exception to this is when the situation is extreme and (by objective standards) largely uncontrollable. In extreme, uncon-

trollable situations like the death of a family member, the surrendering and deferring styles are often the most adaptive. When nothing can be done to prevent or undo the event, surrendering control provides an overwhelmed person with relief.

Spiritual Explanations of Positive Psychology

Deepak Chopra, M.D., is a best-selling author, teacher, and founder of the Chopra Center for Wellbeing. In his book *Nine Keys to Lasting Happiness*, he writes *according to the ancient philosophy of the Vedanta, there are two types of happiness. The first comes from things turning out the way we'd like them to,* i.e.*, getting what we want. We say, "I'm happy because… because I have family and friends, because I got a promotion, because I have money and security." This kind of happiness is inherently fleeting because it depends on external reasons that can be taken away from us at any time. The second type of happiness, in contrast, is a state of being, not something we do or achieve. It isn't dependent upon our mood or outer circumstances. Real happiness comes from having an unassailable connection to the deep state of unbounded awareness at our core. This state of being is our own inner joy that expresses the exuberance and wonder of being alive at this moment; it is our own self-luminous essence made conscious of itself.*

The nine keys to lasting happiness are:

1. Listen to your body's wisdom, which expresses itself through signals of comfort and discomfort.
2. Live in the present, for it is the only moment you have. Don't struggle against the infinite scheme of things; instead, be at one with it.
3. Relinquish your need for external approval. There is great freedom in this realization.
4. When you find yourself reacting with anger or opposition to any person or circumstance, realize that you are only struggling with yourself. When you relinquish this anger, you will be healing yourself and cooperating with the flow of the universe.
5. Know that the world "out there" reflects your reality "in here." The people you react to most strongly, whether with love or hate, are projections of your inner world.
6. Shed the burden of judgment—you will feel much lighter. Remember that every person you forgive adds to your self-love.
7. Support your body and mind by giving it the most nourishing food, experiences, and environment. Your body is more than a life-support system. It is the vehicle that will carry you on the journey of your evolution. Don't contaminate your body with toxins, either through food, drink, or toxic emotions.
8. Replace fear-motivated behavior with love-motivated behavior. Fear is the product of memory, which dwells in the past.
9. Understand that the physical world is just a mirror of a deeper intelligence. Intelligence is the invisible organizer of all matter and energy, and since a portion of this intelligence resides in you, you share in the organizing power of the cosmos. Living in balance and purity is the highest good for you and the Earth.

Neural Basis

Happiness, pursuing that which we enjoy, appears to be neuroscience based as written by Drs. Higgins and George in their book, *The Neuroscience of Clinical Psychiatry: The Pathophysiology of Behavior and Mental Illness* [15]. Happiness is hardwired and closely fluctuates around a genetic "set point." In the context of positive psychology, a meaningful life goes beyond the simple pursuit of pleasure from a biological perspective. We humans have what is called hedonistic hot spots (e.g., in the orbitofrontal cortex, the amygdala, and the ventral pallidum). "Meaningful" incorporates the whole function of the brain. Neuroimaging studies have shown that meditation results in an activation of the prefrontal cortex, an activation of the thalamus and the inhib-

itory thalamic reticular nucleus, and a resultant functional deafferentation (resulting in the reduction of distracting stimuli to the striate cortex) of the parietal lobe. The neurochemical change as a result of meditative practices involves all the major neurotransmitter systems. The neurotransmitter changes contribute to the amelioration of anxiety and depressive symptomatology and in part explain the psychotogenic property of meditation (i.e., the ability to produce altered levels of consciousness that induce wellbeing). This overview highlights the involvement of multiple neural structures and the neurophysiological and neurochemical alterations observed in meditative practices (Mohandas). Compared to the study of negative emotions such as fear, the neurobiology of positive emotional processes and the associated positive affect (PA) states have only recently received scientific attention. Biological theories conceptualize PA as being related to (i) signals indicating that bodies are returning to equilibrium among those studying homeostasis, (ii) utility estimation among those favoring neuroeconomic views, and (iii) approach and other instinctual behaviors among those cultivating neuroethological perspectives. Indeed, there are probably several distinct forms of positive affect, but all are closely related to ancient subcortical limbic brain regions we share with other mammals. There is now a convergence of evidence to suggest that various regions of the limbic system, including especially ventral striatal dopamine systems, are implemented in an anticipatory (appetitive) positive affective state. Dopamine-independent mechanisms utilizing opiate and GABA receptors in the ventral striatum, amygdala, and orbital frontal cortex are important in elaborating consummatory PA (i.e., sensory pleasure) states, and various neuropeptides mediate homeostatic satisfactions.

Religious Perspectives

Hinduism

Sanatana dharma (sana~tan—all human being, Dharma—duties) and vedic philosophy (through the 4 books—Rig veda, Yajur veda, Atharva veda, and Sama veda) preached ways of life, pre-religion, which were considered more spiritual than the structured religion. Yoga, meaning "union," was one of the pathways to teach "union with higher consciousness" during this vedic time. The higher consciousness was later also known as "God," who is formless, omnipresent as light [16]. Thus the concept encouraged every individual to be a "Siddha or Buddha," both meaning an individual who has gained enlightenment, through merging with higher consciousness, with meditative and yogic practices, and thus creating a community of higher collective consciousness (https://www.thehansindia.com/hans/opinion/news-analysis/essence-of-vedas-and-upanishads-681164). The concept of collective consciousness can be described as "Vasudeva Kutumbha," which in literal translation would mean "World family" [16]. Hinduism, as a form of religion, is based on the Vedic philosophies and Sanatan Dharma, based more on the scriptures from Manusmriti (Manu—Human; Smriti—laws). The various scriptures depicted every being as a "God" and thus Hinduism believes in having 36 crores of Gods and Goddesses. The rituals and procedures were eventually created to support and pray for these Gods and Goddesses. The Manusmriti "Laws of Manu" became the rule book for people to follow Hinduism as religion [41] (https://constitutii.wordpress.com/wp-content/uploads/2014/06/the-laws-of-manu.pdf). Although Hinduism is considered rich in terms of scriptures like Puranas, Upanishads, Ramayan, Mahabharat, Bhagvad Gita, and the knowledge of human life imparted through them, it has been divided into various practical applications, around the world. Hinduism's focus has revolved around the concept of "Dharma," known as duties and "Karma" known as deeds. The duties are always considered toward self, one's family, one's community, one's country, and one's world. Whereas the deeds are the actions that an individual would do on a day-to-day basis, for oneself and for everyone else. The teachings focus on having purity in dharma and karma, and "God's" way of managing human destiny, based on whether the duties and deeds were done positively or negatively.

Hinduism has been known to be inclusive and not to believe in conversion of religions. Most of the festivals and holidays in Hinduism are based on the concepts of increasing positivity and elimination of negativity from life. Whether people celebrate Diwali or Holi, the stories revolve around victories of good over evil, positivity over negativity [38].

Indian psychology recognizes three levels of information processing, at the level of the brain, mind, and consciousness [17]. Classical Indian psychology considers the human person as "consciousness embodied." In Hindu, the "Mantra for Positive Thinking" is: *Life is the manifestation of our own thoughts*. Positive thoughts bring about positive developments in life and bring success to individuals. Hindu mantras work with the subconscious mind and transform the potentials of people. Chanting a powerful mantra can encourage a lot of positive thoughts. Mantras are connected to the subtle forces in life. They help connect to the superhuman powers in the cosmos. Chanting a chosen mantra repeatedly as per the procedure advised can help reap the power of the mantras that will result in a positive change in the attitude, tendency, capabilities, confidence levels, and ability to endure in life.

Hinduism propels positive psychology concepts of human wellbeing and happiness. The community level of following customs, traditions, and rituals is known to bring feelings of togetherness, and a source of elimination of negativity from life [37].

Buddhism

Buddhism, also known as Theravada Buddhism, in earlier concepts, originated, post Vedic period, in Northern India several centuries before the Christian era. The main texts of Buddhism are written in Pali language and are called "canon." There are many concepts in Buddhism that are similar to the Indian pre-religion philosophy. Texts talk about Buddha as being a skillful doctor—he may break the bad news of our suffering, but he also prescribes a proactive course of treatment. In this metaphor, the medicine is Buddha's teachings of wisdom and compassion known as *Dharma*, and the nurses that encourage us and show us how to take the medicine are the Buddhist community or *Sangha*. The illness, however, can only be cured if the patient follows the doctor's advice and follows the course of treatment—the Eightfold Path, the core of which involves control of the mind. In Buddhism, this treatment is not a simple medicine to be swallowed, but a daily practice of mindful thought and action that we ourselves can test scientifically through our own experience. Meditation is, of course, the most well-known tool of this practice, but contrary to popular belief, it is not about detaching from the world. Rather it is a tool to train the mind not to dwell in the past or the future, but to live in the here and now, the realm in which we can experience peace most readily. The word meditation is derived from the Latin verb meditari, meaning "to think, contemplate, devise, and ponder" [9]. The actual practice differs from belief to belief; but singularly it focuses on training the mind to gain resilience, strength and a state of "nothingness." Scientific research on meditation clearly indicates that brain circuits important for attention and emotion can be altered by the practice of meditation. We are able to get rid of all the rancor and rag from our system, thereby reclaiming peace and happiness.

Reading/hearing scriptures, positive literature, and associating with the wise and holy (i.e., being like God) will help us cultivate spiritual awareness and acquire a profound understanding of life and its purpose or function. This was the experience of Ignatius when he was recuperating in his home reading books on the life of Jesus and the saints. Life is governed by specific and infallible natural laws like the cause-and-effect principle which need to be comprehended and adhered to. We reap what we sow, as the adage goes. When we embrace the awareness that each individual carries the onus of his or her karmic account and also that we reap accordingly, then our emotions become less turbulent.

In this process of self-evolvement, the efficacy of meditation is paramount. The golden moments of supreme quietude illumine the path to the sublime inner voyage. As we proceed in meditation,

we are able to connect with our inner core gradually; the unexplored and inaccessible territories become more familiar. Our unconscious repressions, grouses, and hurts buried in the darkest crevices of the unconscious mind start to reach the surface. Over time, the intense power and purity of regular mediation remove all toxins, resulting in a serene, happy, and wholesome life [16]. Buddhist psychology of enlightenment provides some relevance to modern psychology, through the practice of meditation. Meditation is considered as an essential aspect of an individual's religious endeavor. Buddhism brings about several behavioral change strategies, which are similar to the modern lifestyle and behavioral modification techniques [29].

Sikhism

Sikh means "disciple" and the prophets are called "gurus," which means teacher. The Guru, Nanak Dev, founded Sikhism (Called Sikhi) in 1469 AD. Sikhism concepts follows the main scripture called "Guru Granth Sahab"—literally meaning "Teacher – Book – Sir." This book or Granth has contributions from many teachers and scholars. The scripture is unique, as it has contributions from Sufis, Hindus, and Muslims as well. It also makes references to the great Indian epics Ramayana and Mahabharata. Sikh theology relies on a monotheistic approach. The final reality is a unity with God, and as God is without qualities, truth becomes an inadequate description. Guru in this context must be an enlightened messenger of God, and this approach has been used in psychotherapy in the Indian context (Neki, 1973). The text describes five ways to achieve salvation: listening (suniyai) to the spiritual hymns (Bani), obeying (mannai) these hymns, reciting the name of the Lord (Naam), meditating and subjective experience of the name, and with the company of saints. These concepts are considered cognitive reframing. It is thought to bring an intuitive balance, described as a state of mind where understanding, peace, and tranquility are present along with true devotion to God, that is known to lead toward the final state of happiness. This final state of happiness is called Samadhi. A mind-body connection has been clearly stated to exist in some verses in the text, if the mind is polluted, then the body is polluted as well, and the tongue is polluted too [35]. The Universality of Sikhism encourages gender equality, belief in science, acceptance of other religions, meditation, belief in democracy, and spirituality of everyday life. Thus, inculcating positive psychological traits in Sikhism. Various studies have shown that those clients receiving religious psychotherapy showed significantly more rapid improvement in anxiety symptoms than those who received supportive psychotherapy and drugs alone [18, 19]. The notion of religious values and spirituality is thus inherent in the community, and feelings of unhappiness, sadness, and difficulties are dispelled using prescribed rituals and religious practices. The strength of the family and religious values can be a positive advantage in forming therapeutic alliances.

Jainism

Jainism's origin is credited to Lord Mahavira. This religion is considered formal and rejuvenating. The principles of Jaina religion are: (1) Self-control, which includes: (a) Control over physiological instinct of hunger and sex; (b) control over desires; (c) control over emotions; (2) meditation; (3) introspection; (4) concentration; and (5) healthy interpersonal relationship. The principles of Jaina religion contribute to Positive Mental Health. Jainism believes in the concept of soul, as being infinite, in four ways, and that the natural characteristics of soul come to full manifestation in the state of "salvation." The soul according to Jainism, consists of four infinities, that is, Infinite Knowledge (*Ananta Gnana*), Infinite Intuition (*Anata Darsana*), Infinite Happiness (*Ananta Sukha*) and Infinite Potency (*Ananta Virya*). These powers of the soul are neutralized by "Karmic" influences, when in the state of "spiritual bondage engrossed in wordly

affairs." Jainism has prescribed two separate ways of practicing religion, one by saints and monks (known as "*shramanas*") who live separately in a holy place, and the other by householder humans (known as "*shravakas*") living with their families. A *shravaka* is one who does his worldly duties and, in addition, follows the doctrine of Jainism. His primary goal is to do his duties as an individual toward his family, toward his society, toward the State, etc. Secondarily, he also practices Jainism. A *Shravaka* follows 12 rules of conduct (*vratas*). He follows these 12 *vratas* to whatever extent possible, the goal being to follow them to the maximum extent without jeopardizing his worldly duties. It is followed that these 12 vratas will help with positive psychology concepts of self-control, help in interpersonal relationship [39].

Islam

Islamic scholar Abu Amina Elias reported on June 4, 2016, "In the Name of Allah, the Gracious, the Merciful" [20] that thoughts have a powerful ability to determine our feelings and emotional states and ultimately affect how we behave, for better or worse. Islam teaches us to direct the act of reflection (*tafakkur*), or deep thought, toward the signs of Allah, the names and attributes of Allah, to his blessings and wonders, to hope in the Hereafter, and to optimism. By controlling our thought processes in a positive manner, we can increase the effectiveness of our prayers and worship as well as relieve ourselves from the anger, depression, and anxieties that worldly thoughts induce. Contrary to popular belief, we have control over which thoughts we choose to follow. We may not have a choice over which particular thought occurs initially in our minds at a given time, but we do have a choice to either ignore it or pursue it. Our voluntary thoughts are nothing more than inward statements. Hence, the rule is that we should only engage good thoughts or keep our minds silent. Abu Hurairah reported: The Messenger of Allah, peace and blessings be upon him, said: وَمَنْ كَانَ يُؤْمِنُ بِاللهِ وَالْيَوْمِ الْآخِرِ فَلْيَقُلْ أَوْ خَيْرًا لِيَسْكُتْ (Whoever believes in Allah and the Last Day, let him speak goodness or remain silent). Sunan al-Tirmidhī 1987, Grade: Sahih also reported "Positive thoughts are those that produce good feelings, good deeds, peace of mind, gratitude, tranquility, contentment, and other positive emotional states. These are truthful thoughts about Allah: hope in the hereafter, the prophets, our blessings, good deeds, and so on. They produce wisdom and enlightenment in the heart. Negative thoughts are those that produce bad feelings, anger, envy, jealousy, hatred, anxiety, depression, and other negative emotional states. These are thoughts about the world, our wealth, our status, people we do not like or who have wronged us, and so on. The cause of these thoughts is an attachment to the delusions of worldly and materialistic life that cloud the heart and prevent its purification."

Sufism

Sufism, based on the Koran and the teachings of Prophet Muhammad (610 CE), was influenced in its formation by Christian asceticism and Hinduism. Its origins are traced to the eighth and ninth centuries. Sufis have been known for a rich tradition of literature, poetry, storytelling, and using clever metaphors in stories, music, and dance. Sufism regards love, faith, experience, and knowledge as central concepts in resolving our existential dilemma to ultimately understand and experience the divinity in oneself or the union of self with the Supreme through a process of progressive *lifting of veils*, which hide true beauty and knowledge. Rumi regarded love as the "creative force in nature" (Arasteh 1965, p. 10); thus, love is the basis for all creation and creativity. In psychological parlance, seeking knowledge can be described as progressive problem-solving to reveal the "hidden treasure" behind veils—true knowledge in the form of unity with essence. Sufis reject the dogma of a single path to salvation. They believe using such strategies as breathing, meditation, music, and dance [21].

Bahá'í Faith

The Bahá'í Faith offers an interesting and challenging ground for psychologists and psychiatrists: as the youngest monotheistic world religion (founded in 1844) [14], it is confronted with the challenge of individualism after millennia of collectivism in religious history; it has one of the most culturally diverse communities in the world (Bahá'ís live in more than 200 countries); its aim is the unity of humankind in diversity; harmony between science and religion is one of its main principles; and the existence of thousands of pages of original and authentic scripture and biographical literature offers the unique possibility of independent investigation—just to name a few aspects which make the encounter between psychologists and this religion interesting (from Peseschkian 2018) [14].

Based on a positive conception of human beings, "…regard man as a mine rich in gems of inestimable value. Education can, alone, cause it to reveal its treasures and allow mankind to benefit therefrom" (Bahá'u'lláh, 1994) [12]. The Bahá'í teachings offer a very positive and encouraging outlook toward life:

> As to spiritual happiness, this is the true basis of the life of man, for life is created for happiness, not for sorrow; for pleasure, not for grief. Happiness is life; sorrow is death. Spiritual happiness is life eternal. This is a light which is not followed by darkness. This is an honor which is not followed by shame. This is a life that is not followed by death. This is an existence that is not followed by annihilation. This great blessing and precious gift is obtained by man only through the guidance of God. (Abdu'l-Baha 1985) [11]

This important realization—that true happiness comes from turning your efforts toward serving other people—resonates throughout the Bahá'í teachings. This quote gives some indication of the Bahá'í view of the value of service as it relates to our inner joy and happiness: "Be not the slave of your moods, but their master. But if you are so angry, so depressed and so sore that your spirit cannot find deliverance and peace even in prayer, then quickly go and give some pleasure to someone lowly or sorrowful, or to a guilty or innocent sufferer! Sacrifice yourself, your talent, your time, your rest to another, to one who has to bear a heavier load than you – and your unhappy mood will dissolve into a blessed, contented submission to God" (quoted by Langness 2017) [13].

Confucianism and Taoism

Confucianism is known to be responsible to shape "chinese culture." Confucius believed the virtues are for helping people live harmonious and happy life. The pillars of Confucianism are composed of virtues including *Zhong* (loyalty), *Xiao* (filial piety), *Ren* (benevolence), *Ai* (affection), *Xin* (trustworthiness), *Yi* (righteousness), *He* (harmony), *Ping* (peace), *Li* (propriety), *Zhin* (wisdom), *Lian* (integrity), *Chi* (humility/shame). It follows a philosophy, where human's life would be a "good life" if they follow their role dutifully, while discouraging one's own self-centered pursuit. [28]

Taoism offers insights into duality or polarity in nature. It identifies that problems and stress disappear, when negative and positive are seen as "integrated whole." Problems in life are known to be secondary to ignorance about the way of nature. According to this belief in duality, one's strength may contain "seed of self-destruction." While the "strength may be hidden in one's weaknesses." Taoism does not consider it to be "wise" to exclusively focus on developing individual level strength or maximizing positive experiences, as it increases the feeling of craving for success and happiness, that could lead towards moral depravity and life's destruction. Taoism considers through duality develop acceptance, effective adaptation, flexibility around negative experiences in life, teach happiness by addressing ways to feel free from stress and achieve contentment. Taoism also considers contentment as a natural way of life that leads to humility, developing an attitude of "surrendering to nature," resisting interference in way of nature and avoid control of situation thus letting nature take its course, being selfless for well-being of the humanity, thus leading to health and happiness.

Thus "learning the wisdom of way" and meditation "teaching the art of doing nothing," per Taoism, offers happiness and contentment in life [28].

Judaism

Judaism is definitely an optimistic religion, and positive thinking plays an important role in a Jew's world view in general and service of God in particular. All the stories in the Torah are positive. Individuals or the Jewish nation may have experienced difficulties or suffered setbacks, but ultimately, they succeeded and good was achieved and prevailed. This is true regarding Adam's repentance and the continuation of humanity, the preservation of Noah and his descendants through the flood, Abraham's spreading the message of God to all mankind, the tribes' redemption from Egypt, and the Jewish people's receiving the Promised Land. The fact that all of these wonderful things happened not in the context of serenity, stability, and prosperity but rather on the backdrop of great personal and national obstacles and hardships illustrates that Judaism's optimism is not only of a "natural" sort, but even when this kind of optimism is against all odds.

There is no "tragedy" in Jewish literature, nor is there "fatalism" in Jewish philosophy. Even the suffering Job, tutored and elevated by his hardships, rose to true greatness in the end. The promising messages of the Prophets and the novel idea of the Messianic Era in Judaism also demonstrate its essential optimism. A religion which maintains such a glorious view of the future despite the often-dire reality of the present, particularly regarding the Jewish people itself, certainly encourages positive thinking. Prayer is another example of Jewish optimism. One need not accept imperfections of the present. In fact, the word for prayer in Hebrew, *tefilla*, connotes wrestling. In prayer, one wrestles with oneself, with one's reality, and even with God in order to change things for the better. That being said, Jewish positive thinking also enables one to accept an imperfect present if need be. Or rather, Jewish optimism extends beyond hoping for a brighter future to include illuminating what seems to be a dismal present.

If after making responsible efforts to improve our reality, things don't get "better," Judaism teaches not just that one must accept (at least temporarily) his lot but also that this reality, in so far as it is the Will of God (slowly being revealed), is actually the best possible reality. This extending the idea of "making the best of a bad situation" to "seeing what seems to be bad as the best situation" is exemplified by a famous story regarding Rabbi Akiva. Rabbi Akiva was on his way and needed lodgings for the night. When he entered a town asking for hospitality, he was summarily denied it. Exclaiming: "Whatever God does is for the best," he set up camp in the nearby forest, lighting a candle and making provisions for his rooster and donkey, intending to get an early start in the morning. During the night, a wind came and blew out the candle leaving him in complete darkness. He exclaimed, "All is for the best." A cat came and ate the rooster preventing his early rise—"All is for the best." A lion came and devoured the donkey taking his source of transportation—"All is for the best." In the morning he saw that the town had been invaded and looted by robbers and all the inhabitants had been killed. When Rabbi Akiva considered that had he been admitted to the town or, after having camped in the woods, had the candle been lit, he would have been seen and had the cat meowed or the donkey brayed, he would have been heard, he once again justifiably exclaimed, "All that God does, He does for the best!"

An interesting idea to ponder is whether our thoughts actually have an effect on reality, such that pessimism breeds a negative actuality; while optimism actually creates a positive reality. The mystical and Hasidic teachings of Judaism are replete with the idea that not only what we do and say has an effect for good or for bad on the physical and spiritual worlds but even something as subtle and intangible as thoughts have such an effect. And in truth, since God thought existence into being (the statement in the book of Genesis, "Then God said, 'Let there be light.' "being

understood as an expression of God's will, since God doesn't speak), and since mankind was created in the image of God (again, not to be understood literally but rather in our ability to will), then just as God thinks creation, our thoughts can also create reality and it's positively worth thinking positive!" (www.rabbiulman.com).

Rabbi Levi Brackman and Sam Jaffe [22] in an excerpt from their new book *Jewish Wisdom for Business Success: Lessons from the Torah and Other Ancient Texts* write "thoughtful optimism breeds success." People by their nature, optimistic and positive and cheerful and fun to be around, are chosen over negative people when it comes time to win the contracts or make the sale. Torah teachings stress optimism and positive thinking as keys to attracting success and positive outcomes. At the same time, the Torah also stresses that thought itself will not accomplish anything. It is only when thought is combined with action that positive thinking can result in the realization of dreams and the bringing of success.

Christianity

The idea that positive thinking results in positive outcomes can be traced back to the story of Noah and the Flood in Genesis. Peale's most widely read book *The Power of Positive Thinking* [23] was published in October 1952. It came at a time when national views of spirituality, individuality, and religion were shifting and the Cold War was a growing concern for many Americans. These factors, as well as Peale's growing popularity as a motivational public figure and the book's clear prose, propelled *The Power of Positive Thinking* into a self-help book. Peale begins by stating ten rules for "overcoming inadequacy attitudes and learning to practice faith." The rules include the following: picture yourself as succeeding, think a positive thought to drown out a negative thought, minimize obstacles, do not attempt to copy others, repeat "If God be for us, who can be against us?" ten times every day, work with a counselor, repeat "I can do all things through Christ which strengthen me" ten times every day, develop a strong self-respect, affirm that you are in God's hands, and believe that you receive power from God. The next chapter describes the importance of creating a peaceful mind, which can be done through inspirational reading, clearing one's mind, or visualization. Peale continues with how to obtain consistent energy, saying that "God is the source of all energy." The mind controls how the body feels; thus, letting go of negative energy and emotions will give infinite energy through God. Next, Peale speaks of the healing power of prayer and how it will heal physical and emotional problems that arise from negative circumstances. In Chapters 5 and 6 of his book, Peale asserts that happiness is created by choice and that worrying only inhibits it and should be stopped. The next step in thinking positively is to always believe in success and not to believe in defeat because most obstacles are "mental in character." Habitual worrying is the next obstacle to overcome through emptying the mind and positive affirmations. Peale then states that asking God for help can solve one's personal problems and physically and emotionally heal them. In Chap. 12, he states that letting go of anger and embracing a sense of calm can help with physical illnesses, such as eczema. Next, he states that letting positive thoughts in can change one's outlook on life drastically and that practicing relaxation through God's help will lead to a content life. Chapter 15 gives concrete examples on how to get others to like you, including the following: remember names, praise others generously, become a people person, and resolve problems calmly as soon as they appear. Peale then continues with how to overcome heartache—through prayer, meditation, social interactions, and keeping a daily routine. The final chapter restates the importance of reaching out to a higher power for help in living a peaceful and positive life. Peale ends his book with an epilogue encouraging readers to follow his techniques and live more fulfilled lives. Peale writes, "I pray for you. God will help you — so believe and live successfully."

Ignatian Spirituality and Positive Psychology

St. Ignatius of Loyola was the founder of the religious order "Jesuits."[7] He was also the author of the *Spiritual Exercises*, first published in 1548. For Ignatius of Loyola, the meaning of life can be captured in his life motto or mantra, *Ad Majorem Dei Gloriam,* translated as "For the Greater Glory of God." Iñigo de Loyola was born in 1491, the youngest of a family of 13, in the Basque region of northern Spain. Iñigo spent much of his young life aspiring to become the ideal man of that time and age, a courtier and a soldier. The young Iñigo was something of a ladies' man and a real hothead. He sustained battle wounds in the French army in 1521 and necessitated his convalescence at his family castle of Loyola, Guipúzcoa, in Basque region of Spain. On his sickbed, he experienced a deep conversion. "Within him stirred a strange desire – to become like the saints and serve God." Gradually, he determined to turn away from court ambition, to embark on a pilgrimage that was to last years, and took him finally to his studies for the priesthood at the University of Paris. There he gathered a group of friends who, in turn, presented themselves to the Pope as a new religious order. In 1540, the Society of Jesus was officially approved by Pope Paul III. For Ignatius, this kind of peace and happiness is what is being sought out in the "Examen." But it is more nuanced than that as I will explain further. But the goal is that if *unburdened*, we can move on in life.

The following are some takeaway points from Ignatius' life, preaching and writings [5, 6].

1. Ignatius's take on the Christian journey is to insist that it is a movement, an active progress toward a radical decision to live one's life in harmony with Christ's vision and values.
2. Discovering one's true self and being truthful to this self is a dynamic that will lead to further oneness or intimacy with God. This kind of communication, a unique form of spirituality, should lead to further action or service.
3. Ignatian concepts of consolation and desolation (i.e., movements of negative and positive affect) may be analogous to what Seligman describes as the subjective level of human experiences of: joy, satisfaction with life, relaxation, love, intimacy, and contentment. Subjective well-being "is a person's cognitive and affective evaluation" of oneself.
4. Discovering God's will, as described earlier (as one and the same with his own deepest desires or his will), would bring true lasting happiness, joy, satisfaction, peace, and contentment.
5. St. Ignatius: concept of "Examen" of consciousness, a form of meditative prayer where Ignatius learns how to distinguish between internal movements that are "of God" and those that are "not of God."
6. A central theme in Ignatian spirituality is this "battle within us" to do the right thing. It is a *spiritual journey* toward God. Early in his conversion, Ignatius was able to recognize the difference between that which moved him toward God (the consolation he felt when he thought of serving God), and that which moved him away from God (the dry feelings that attended his plans to seek fame). The discerning person, Ignatius believed, could distinguish between these two forces and make the right choices. In Ignatian spirituality this is called "discernment of spirits" [6].

Spirituality and Prayer

Concept of spirituality from vedic philosophy perspective is beyond the constraints of structure as in religionism. It provides the basis to understand the concept of higher power, which is in the form of infinite energy or consciousness. This energy is known to be in all forms of nature, including humans. Vedic and Vedanta philosophy provides tools for human beings to merge with this infinite and thus transcend. It usually operates on individual and community levels by inculcating practices of individual lifestyle modifications, community service, and collective prayers. [16, 40]

Vedanta has been known to affirm: [32]

- The oneness of existence.
- The divinity of the soul.
- The harmony of all religions.

Prayer and Positive Psychology

The purpose and meaning of prayer in the context of the definition of positive psychology is to be able to discover one's own purpose and meaning in this life. It is a journey of relating with a "Thou," the mysterium tremendum. Marcel would describe this Thou as *disponibilité*. The book, *Spiritual Exercises of Ignatius of Loyola,* is the human being's guide to better understand what St. Ignatius calls the *foundational truths of Christian life*: creation as an act of love; human stewardship of creation, sin, and forgiveness; the life and work of Jesus as a paradigm of discipleship; Christ's suffering, death, and resurrection (what Catholics refer to as the Paschal mystery); and, finally, the surrender of human life into the hands of a loving God—first to love the Lord your God, with all your heart, and with all your soul, and with all your mind, and with all your strength and second to love your neighbor as yourself.

So, now, we can understand more clearly what kind of life is "worth living," for Ignatius of Loyola. He described his spirituality as a journey toward God (*The Formula of the Institute*). According to another Jesuit author, Fr. Howard Gray, Ignatius envisioned most everything in life as part of that journey—as created realities that facilitated or impeded its progress. For Ignatius, spirituality is about finding God in all things. His spirituality is based on the knowledge that "other things on the face of the earth are created for man to help him in attaining the end for which he was created." Shridhar Sharma in his paper, *Spirituality, Yoga, Religion and Mental Health*, describes spirituality as: "…concerned with soul and is directed towards the pursuit of personal meaning." Sharma adds: "Soon a spiritual person comes to realize that the self in him is the self in all, thus experiencing 'Godhood'."

Not being happy in this lifetime may be a universal fear or source of anxiety for any human being. For Ignatius, that aspect of life which gives all human beings meaning and purpose in life is desiring and choosing that which is conducive to the end for which we all are created "…to praise, reverence, and serve God our Lord." [6]

Positivity of Religion and Spirituality in Clinical Psychiatry

Though horrible atrocities have been committed in the name of religion throughout history, research suggests that religious and spiritual beliefs have tremendous positive benefits to individuals and societies. Research on religion and coping suggests that the benefits of religion have to do more with how you are religious—your religious style or orientation—than whether you are religious [36]. There are two ways to think about religious style. Pathways have been discussed in the literature through which religion/spirituality influenced positively is by increased social support, enhancing the importance of positive emotions, such as altruism, gratitude, and forgiveness in the lives of those who are religious. In addition, religion promotes a positive worldview, answers some of the why questions, promotes meaning, can discourage maladaptive coping, and promotes other-directedness [27].

Listed below are some of the benefits of religion reviewed in the Peterson and Seligman Handbook (2004) [26]:

- Several studies have shown fairly consistent relationships between levels of religiosity-enhanced outcomes of depressive disorders, traumatogenic conditions, substance abuse disorders, and others.
- Religion has been found to enhance remission in patients with medical and psychiatric disease who have established depression.
- Negative religious coping (being angry with God, feeling let down), endorsing negative support from the religious community, and

loss of faith correlate with higher depression scores.
- Studies demonstrated lower levels of anxiety among more religious people. With regard to religious involvement in substance abusers, 90% found less substance abuse among the more religious. In some subjects preciosity caused more guilt and more anxiety. Among young people in particular, being religious is associated with reduced smoking, drug, and alcohol use.
- Patients reported that religion lessened psychotic symptoms and the risk of suicide attempts, substance use, non-adherence to treatment, and social isolation.
- Religion is associated with higher levels of obsessional personality traits but not with higher levels of obsessional symptoms.
- Young people who engage in religious practices (like going to church) are also more likely to have better grades and delay having sex.
- Being religious has positive benefits for relationships. People who actively participate in religious activities and who view religion as important are less likely to experience conflict in their marriage and more likely to perceive their spouses as supportive.
- Religious parents are also more likely to parent consistently and less likely to have highly conflictual relationships with their teenagers.
- Religious beliefs and practices are predictive of other virtues such as altruism, volunteerism, kindness, and forgiveness. Similarly, churches that actively promote displays of these values (especially volunteerism and philanthropy) are associated with community well-being.

Summary/Conclusion

The important correlation of happiness according to Park is social in nature [4]. In contrast to the modest demographic correlates of happiness and well-being, being religious is one robust correlate (in addition to number of friends, being grateful, being part community or marital union, being employed, etc.). Whether one follows the religious rituals or the spiritual lifestyle pattern, the journey is envisioned to inculcate feelings of happiness, peace, hope, faith and ultimately positive health. But religion is defined here as a specific tradition or, in Ignatius' case, a certain way of proceeding. His way of proceeding is based on his own life story. His journey toward his true self led him to this life that he considered "the fullest." His method for achieving this realization of the true self was the *Spiritual Exercises*. The intention of the *Spiritual Exercises* is that of discovering and following this desire or choice that is conducive to our end: the end being following Christ's example. Ignatian meditation is not a form of rational (logical) analysis, nor does it seek scientific accuracy. It is rather a manner of experiencing and then discerning, that is, evaluating, the spiritual affects of (what Ignatius called) "consolation" and "desolation." We can only discover our true selves through living a spiritual life. This spiritual life is our bridge; it is our way of communicating or relating with God (who is also actively trying to communicate with us in this world). This relationship is nourished and strengthened by prayer, specifically the "Examen of Consciousness." Through the Examen, we are able to distinguish between what is from God and what is from "the enemy of our human nature" [6]. Discovering what is from God is synonymous with finding our true selves. The evangelist John succinctly articulates this thesis of "finding our true selves as we find God," in his letter to the early Christians: "We belong to God, and anyone who knows God, listens to us, while anyone that does not belong to God, refuses to hear us. This is how we know the spirit of truth and the spirit of deceit" (1 John 4: 6–7) [10]. This ultimate consolation or subjective well-being as Seligman describes it is not something that is readily available in this material world; one has to seek it out patiently, fervently, and actively. A prayerful life requires a strong desire. Ignatius would later realize that even this desire is a gift that only God can give. This desire could be what Csikszentmihalyi was trying to describe with the concept of *flow*. This desire to *find God in all things* is channeled through a spiritual life or a life of prayer. And living a spiritual life will ultimately result in finding

our true selves. Living out our true selves will lead to a happy and joyful life. For Ignatius it is a life that should bring us to service: of being contemplative in action. By the correct use of Ignatian discernment, one can be enabled autonomously to make decisions (to use the motto of the Jesuit order) "for the greater glory of God." It is a guide that in a way simplifies how we should live.

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Positive Psychotherapy as an Existentialism

39

Andre R. Marseille and Erick Messias

Introduction

Despite civilization's technological and medical advances, an observation of the present world reveals a landscape full of economic instability, shifting demographics, and threats of war. Such is the case that humanity has become more desensitized to the basic needs of displaced immigrants and refugees, the deaths of young children in unnecessary captivity, or the growing evidence that the Earth's climate is changing for the worst, posing a host of new and unforeseen challenges. Around the world, new racial and ethnic divisions are developing as some displaced groups assimilate successfully with the norms of mainstream cultures, while others are rejected, "racialized," and labeled as "minority" or disadvantaged brown and black minorities (Bonilla-Silva and [1, 2]). This stands in stark contrast to the fast-moving pace of technology and economic growth, creating thus a severely unbalanced—and unstable—world. In fact, the term VUCA world—standing for Volatility, Uncertainty, Complexity, and Ambiguity—has been proposed to describe the post-Cold War environment. The eclosion of the COVID-19 global pandemic and the resurgence of major wars have emphasized this sense of uncertainty.

Existence in such a world requires that people have a capacity to change, adjust, and adapt to not only their own changing personal needs but also environmental demands and opportunities as well. The notion of having a "career for life" no longer exists in the face of globalization, increased life expectancy, and technological advances. As a result, a growing number of young people experience difficulties "finding themselves." Today, people live in a transcultural world, "shorn of moorings and in alarming disarray, doggedly exposing the extreme states of anxiety and confusion" ([3], p. 339). Many professionals who report the news and cover social media have described these exigencies as collateral by-products of globalization, while others view them as existential crises.

Globalization poses a new set of challenges for the mental health profession. Globalization describes the accelerated integration of ideas, cultures, practices, and resources of sovereign nations into a more centralized existence [4]. As a result, globalization has rendered many existing conceptual therapies inadequate because they can no longer accommodate the rapidly shifting values, multicultural landscapes, and emerging identities of people around the world. Despite the

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_39

darker impulses of human beings as chronicled through world history, positive psychotherapy (PPT) adopts a humanistic, positive, and optimistic view of humankind. In PPT, change is always possible. The German author Hermann Hesse [5] once said, "There is a magic in every beginning."

In existentialist philosophy, this concept is known as *becoming* [6]. According to German philosopher Heidegger [7], *becoming* describes "as far as we know, the human being is unfinished, always responsive to its world, and repeatedly carrying himself forward freshly, until death" (p. 65). According to many Eastern views about nature including Hindu and Buddhist, life is but a series of changes [8]. Existential philosopher Jean-Paul Sartre [9] argued that as long as people lived, they would forever reinvent themselves. Psychiatrist and psychotherapist Viktor Frankl [10] reasoned that it is from this view of existence that renders human beings capable of finding meaning in their lives. Meaning is perhaps the primary concern for existentialists.

This book chapter details how existentialist philosophy and positive psychotherapy can help people develop more insights and meaning into the core issues of life. We discuss the relationship among culture, consciousness, human capacities, and meaning. According to PPT founder Nossrat Peseschkian [11], humans have two innate capacities that underpin consciousness: the basic capacity to love and the basic capacity to know. These basic capacities make up the contents of the unconscious. PPT as an existentialism emphasizes humans' basic capacity to change and develop meaning in life and how one makes these capacities possible.

Culture and Acculturation

Peseschkian likened human capacities to seeds in the ground. Like undeveloped capacities in humans, seeds have an abundance of untapped potential. When properly nurtured (i.e., good soil, rain, and a caring gardener), they blossom. Such is the case with basic capacities. How one's consciousness is shaped, how his/her capacities are developed, is deeply connected to the ecosystem or specifically the culture they grow up in. Man is thus a by-product of culture [12]. Culture is a biopsychosocial trend that is as old as human history itself. Though there are several definitions of culture, in effect, culture takes the form of durable, long-standing beliefs, traditions, and practices that have defined and differentiated various groups of people for over millennia [7]. According to Vontress [6], culture is simultaneously visible and invisible, conscious and unconscious, and cognitive and affective.

In his work, *Social-Class Influences upon Learning*, Davis [13] explains culture this way:

> Culture, may be defined as all behavior learned by the individual in conformity with a group. Culture "teaches" the individual not only to recognize certain phenomena, but also certain symbols of phenomena, and the logical relationships among them. Culture also sets the goals of human problems and teaches the inferences (logic) which people in a particular culture regard as profitable (p. 59–60).

Learning implies a process. As many anthropologists have indicated, all human problem-solving includes cultural learning [13]. Acculturation is a process that psychiatrist Harry Stack Sullivan [14] describes as *consensual validation.* In his model, consensual validation is the process of testing one's cultural identity with the larger world through interpersonal interactions and self-appraisals. It is the process by which a person arrives at a healthy consensus with one or more persons about some aspect of his feelings, thoughts, and interpersonal relationships. This consensus is validated by repeated experiences that emphasize its soundness [14].

In positive psychotherapy, Peseschkian posits that every person, independent of their makeup (i.e., age, sex, race, type, illness, or social abnormalities) or culture, possesses two basic capacities: the capacity to love (emotionality) and the capacity to know (learning) [15] both of which cannot escape the influences of culture, acculturation, and the consensual validation process. Based on culture diversity, people's basic capacities differentiate and lead to a coalescing structure of essential traits (personality) over time. The expression of differentiated capacities

defines each person's uniqueness. As a rule, basic capacities and the capacities that underpin them operate in all cultures. The difference lies only in how some cultures versus others value each capacity and how people from different cultures express them.

Despite these cultural differences, PPT poses two fundamental questions about human existence: (1) What do all people have in common? (2) In what ways are people different? In many respects, both existentialism and PPT argue that culture is the answer to both. Culture connects people intimately and inextricably to experience. In this way, culture parallels experience. Among many things, culture provides people with a lens by which to interpret experience. Further, culture allows people to anchor their experiences in history and basis by which to freely investigate any origin and changing nature of their thoughts, values, beliefs, and decisions [16].

Anthropologist Edward Sapir argued "The true locus of culture is in the interactions of specific individuals" and "in the world of meanings which each one of these individuals may unconsciously abstract for himself from his participation in these interactions" ([17], p. 515). Embedded in the acculturation process is a perpetual self-appraising process that oscillates among factors like culture, experiences, and development.

Positive Psychotherapy

> The smart one wants to know everything, the wise does not need to know everything.
> Nossrat Peseschkian

Originally titled Differential Analysis, Positive Psychotherapy (PPT) became the official title in 1977. Founded by Dr. Nossrat Peseschkian, an Iranian-born medical doctor who became a German citizen and member of the Baha'i Faith until his passing in 2010, he was displeased with the limitations of medical science in the holistic healing of his patients. Undeterred, Dr. Peseschkian began an intensive study of over 20 Eastern and Western cultures. Through his endeavors, he discovered that despite people's cultural differences, people generally displayed the same innate capacities to navigate life. Further, he observed that people also tended to interpret and resolve challenges in roughly familiar ways as well.

Peseschkian describes PPT as a psychotherapeutic method to psychological healing which derives its scientific foundations from psychodynamic and behavioral psychologies and psychosomatic medicine, as well as philosophical inspiration from the writings of the Bahá'í Faith. PPT is essentially humanistic at its core. In other words, PPT views man's nature in a positive light. The term "*Positive*" in positive psychotherapy derives from the original Latin expression "positum or positivus" which means the actual, the real, the concrete [18]. This chapter discusses three significant concepts in PPT: life balance, life energy, and capacities.

The Balance Model

Peseschkian [11] argued that all of existence may be interpreted through four domains of life. The effort (i.e., life energy) one applies to these interconnected and interdependent domains of life will dictate the extent by which he/she will experience life balance. In Fig. 39.1, Peseschkian [18]

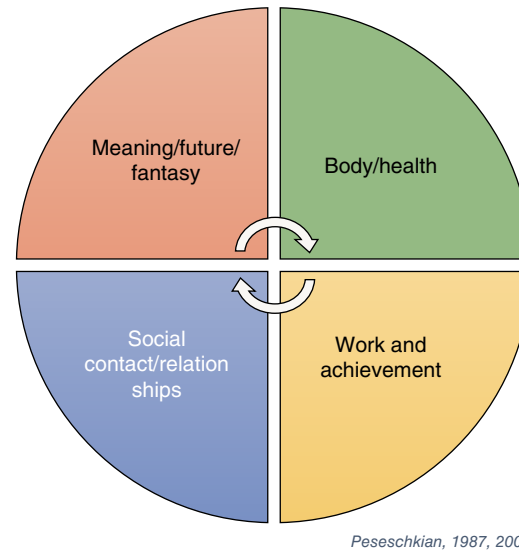


Fig. 39.1 Life balance model [19, 20]

describes the balance model as the four domains that capture all of existence. Balance is about the equal application of life energy one applies across the four life domains: (1) body/health, (2) work/achievement, (3) social contacts/relationships, and (4) future/fantasy and meaning.

Body/Health

This domain refers to the physical body, the five senses, health, and appearance. At the outset, a child's development necessitates contact with its environment. In infancy, this contact initially occurs primarily through the senses. According to Peseschkian [20], the body domain includes the "biological factors which are basic to life: metabolic processes, reflexes, heredity, physical maturation, the functioning of the bodily organs, the functional capability of the senses and the vital needs" (p. 65). In this domain, one is concerned with physical health, appearance, and self-perception.

Work/Achievement

This domain is concerned with achievement and how it is incorporated into the self-concept. Thinking and understanding make it possible for one to solve problems and optimize achievement in a systematic way. Be it work, school, sports, or career, in the most practical sense, the work/achievement domain has everything to do with one's ability to take on tasks and resolve problems. It is a test of reality. This domain has strong implications for self-esteem and self-worth. In this domain, a person is concerned with competition, goals, and self-efficacy.

Contacts/Relationships

This domain represents one's capacity to establish and cultivate relationships including relationships to self and others, to one's partner and family, and with other people. A person's personality is especially influenced by those people closest to them and during the most intensive time of development. According to Sullivan, personality development begins in infancy and extends through early adolescence, growing in significant ways until late adolescence where "there was a tendency for the personality structure a person has developed to persist in its broad, general outlines" ([21], p. 76). The product of this process, Allport describes, is "a man working within a frame of choice, not destiny. The way a man defines his situation constitutes for him its reality" ([22], p. 84).

Future/Fantasy/Meaning

This domain refers to what some religious traditions call inspiration and what some psychological theories call intuition. Here is a place where fantasies go beyond the immediate reality and acquire meaning and future orientation. In this domain, notions of imagination, goals, values, and virtues are considered. One may think of this domain as their life compass that guides them spiritually, emotionally, and psychologically. This domain feeds into one's capacity to think creatively or "out of the box" to address life challenges.

Life Energy

Several cultures around the world have described, over hundreds of years, life energy beliefs. In many ways, life energy has a universal quality. For example, life energy exists in many traditions, such as qi (chi) in China, prana in India, pneuma in Ancient Greece, spiritus in Latin, ruach in Hebrew, and vitalism in philosophy. Take for instance the G-field; it is an expression coined by the English physicist Oliver Joseph Lodge (1851–1940), used in his theory from the early twentieth century about a life force permeating the universe. In his view, matter was a concentrated form of this type of energy.

According to Peseschkian, everyone operates from 100% life energy. Though life energy is considerably more, one may think of life energy as effort. When a therapist asks a client to consider his life balance, he is asking him/her to consider how much life energy they apply to the four domains of existence. The concept of life balance implies a simple arithmetic. Ideal life balance is an equal distribution of life energy (25% × 4) among all four domains. This implies that one is applying a

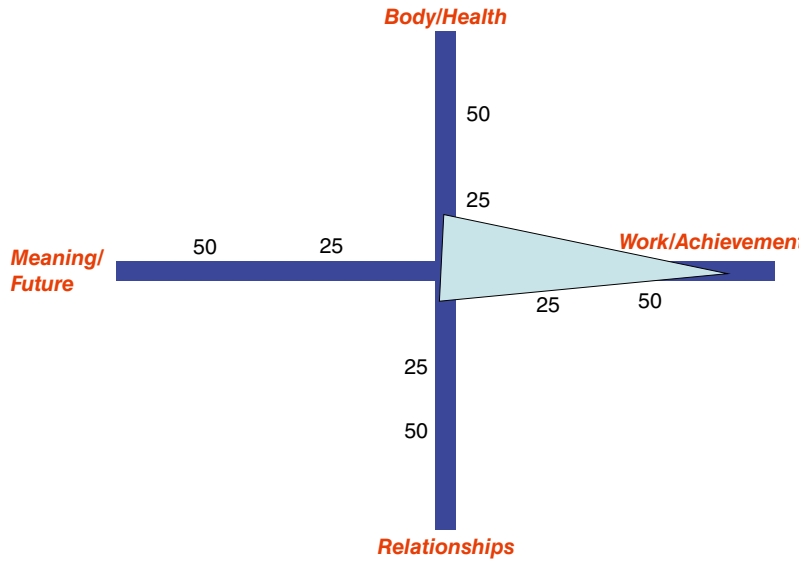


Fig. 39.2 Life energy distribution model [18]

balanced effort to his health, his career, his family and friends, and his goals and values. Should his life balance model reflect an imbalance such as the example in Fig. 39.2 [19, 20], what is causing the imbalance? The nature of this discussion creates the basis for some fertile therapeutic discussions.

In Sullivan's [14] *Interpersonal Theory of Psychiatry*, he describes the concept of energy as essential to interpersonal existence. Sullivan conceptualized personality as an energy system, with energy existing as either a tension or energy transformations. Tension is energy created from an unmet need (food) or desire (touch). Energy displacements literally involve the transformation of potential energy (tensions) into actual energy displacements (behavior) for the purpose of satisfying a need or reducing anxiety. Whereas tensions are helpful or conjunctive when satisfied, they can produce harmful anxiety when not satisfied. Sullivan used the words "integrate" and "integration" to describe the energy displacements that draw people together or push them apart. As Sullivan noted, "everything that happens to us that gets any sort of notice from us—conscious or unconscious, witting or unwitting—fits into the theory of the development and refinement of integrating tendencies or the interpersonal processes that characterize us" ([14], pp. 69–70).

Capacities

From the perspective of PPT, in any interpersonal relationship, what is factual are not just conflicts but possibilities that can be developed. However, these physical, mental, and spiritual capacities need to be differentiated to be developed. Here culture and consensual validation play critical roles. Per Peseschkian [15], "the two basic capacities, to know and to love, are the foundation upon which the capacity for meaning develops" (p. 43). He further explains that human's basic capacities manifest themselves in the world as social norms, family norms, and patterns of behavior, value systems, and sources of internal and external conflict. PPT makes a distinction between basic capacities and actual (primary/secondary) capacities. The basic capacities are underpinned by primary and secondary capacities that are considered concrete contents of both the unconscious and psychosocial interactions.

Capacity to Love

The basic capacity to love is the domain of emotionality, feelings, and drives [15]. The capacity to love is initially expressed instinctually. In other words, people express their emotive needs

and desire them to be met by a nurturing caretaker or partner or friend. This is a simple reciprocal process of emotionality [14]. One's capacity to love is the central form of expression in interpersonal relationships. It is the ability to love and be loved [14]. When one loves and is loved in return, he/she accepts something from another and is accepted in return. When this capacity is developed, other (primary) capacities within this spectrum are developed. Table 39.1 lists the primary capacities thusly.

Primary and secondary capacities are the actual capacities. In other words, they are actual because they are actualized in daily life and take many forms within daily interpersonal living. They are not mere hypotheticals or abstractions like the id, ego, or superego concepts but rather dynamic interacting realities. These dynamic capacities, which have

Table 39.1 Primary capacities to love

Capacity to love	Definition	Possibilities
Love	Strong affection for another arising out of kinship or personal ties; unselfish loyal and benevolent concern for the good of another	Worship, adore, appreciate, fondness, devotion, ardor, tenderness, passion
Modeling	To produce a representation or simulation; to construct or fashion in imitation of a particular model; being a usually miniature representation of something	Demonstrating, sculpting, fashioning, patterning, teaching, learning
Servitude	A condition in which one lacks liberty specially to determine one's course of action or way of life	Bondage, serfdom, vassalage, dependency, subordination, subservience
Doubt	To call into question the truth of, to be uncertain or in *doubt* about; to consider unlikely; uncertainty of belief or opinion that often interferes with decision-making	Hesitation, uncertainty, reservation, distrust, suspicion, skepticism
Trust	*Assured* reliance on the character, ability, strength, or truth of someone or something; to hope or expect confidently	Reliance, expectation, hope, belief, conviction, presume, (despair)
Sexuality	The quality or state of being *sexual*; expression of sexual receptivity or interest especially when excessive	Self-esteem, worth, trust, fidelity, love
Confidence	A feeling or consciousness of one's powers or of reliance on one's circumstances; faith or belief that one will act in a right, proper, or effective way	Self-reliance, poise, sureness, buoyancy, faith, assertion, (uncertainty)
Unity	A condition of harmony, the quality or state of being made one	Harmony, agreeable, according, unison (disarray)
Time	A nonspecial continuum that is measured in terms of events which succeed one another from past through present to future	Rhythm, moments, phase, era, tempo, while
Contact	The apparent touching or mutual tangency of the limbs of two celestial bodies or of the disk of one body with the shadow of another during an eclipse, transit, or occultation	Connection, interaction, communication
Patience	The capacity, habit, or fact of being enduring, persisting, persevering	Endurance, tolerance, persistence, fortitude, perseverance, (impatient, temperamental)
Faith	Allegiance to duty or a person, strong belief or trust in someone or something, belief in the existence of god; strong religious feelings or beliefs	Reliance, conviction, belief, fidelity, constancy, allegiance, (doubt)
Hope	To cherish a desire with anticipation: To want something to happen or be true, to desire with expectation of obtainment or fulfillment.	Expectation, aspiration, wish, yearn, optimistic, faith (despair)
Source	Merriam-Webster online dictionary	Source: Peseschkian [19]. In Merriam-Webster.com Retrieved May 8, 2019, from https://www.merriam-webster.com/dictionary/hacker

been referred to as virtues in philosophical and religious literature, are inherent and essential to the human condition. Within our innate basic capacity to love, one learns to trust and how to mitigate basic existential and interpersonal anxieties like love, death, loneliness, trust, security, etc.

Capacity to Know

According to Peseschkian, man's nature is to ask questions and seek answers to them. The capacity to know is mans' ability to learn and to teach others what he has learned [20]. This capacity has deep implications for one's sense of self-worth, esteem, efficacy, and overall purpose. Peseschkian [19] reasons, "one's capacity to know is not only conditioned by social experiences but by his/her unconscious mind" (p. 99). The capacity to know initially begins by differentiating self from nonself as the cognitive structures mature. Each person develops uniquely, dependent upon the individual's own inherent characteristics, family, environment, society, culture, and worldview of others we learn from and react to. We begin by testing what we learn about our world and the assumptions we make about how this world works—which capacities are enforced in the family, which are left undeveloped, or which are repressed. See Table 39.2 for a complete list of secondary capacities.

Table 39.2 Secondary capacities to know

Capacity to know	Definition	Possibilities
Punctuality	Being on time, prompt	Reliability, regularity, time management (lateness, tardiness)
Orderliness	The quality or state of being clean, sequential, appropriate, organized	Neatness, tidiness, uniformity, symmetry, (disorderliness, messy)
Cleanliness	The quality or state of being clean: The practice of keeping oneself or one's surroundings clean	Sanitary, hygiene, purity (dirtiness)
Justice	The establishment or determination of rights according to the rules of law or equity: The quality of being just, impartial, or fair	Fairness, impartiality, reasonableness, integrity, lawful (unfairness, injustice)
Diligence	Steady, earnest, and energetic effort: Persevering application; earnest and persistent application of effort especially as required by law	Assiduousness, industrial, meticulous, attentive, careful (carelessness)
Truth/ honesty	The body of real things, events, and facts, the property (as of a statement) of being in accord with fact or reality, fidelity to an original or to a standard	Uprightness, morality, goodness, rectitude, honorable, sincerity, integrity (immorality, insincere)
Reliability	The quality or state of being reliable, dependable, ability to produce the same results	Dependability, consistency, steadfastness, trustworthiness (untrustworthiness)
Thrift	The quality of being frugal, parsimonious, careful management especially of money	Frugal, careful, prudent, cautious (extravagance)
Conscious	Having mental faculties not dulled by sleep, faintness, or stupor, perceiving, apprehending, or noticing with a degree of controlled thought or observation; capable of or marked by thought, will, design, or perception	Aware, mindful, cognizant, sentient, sensible (unaware, unintentional)
Courtesy	Behavior marked by polished manners or respect for others: Courteous behavior, consideration, cooperation, and generosity in providing something (such as a gift or privilege)	Politeness, considerate, civil, gallant, gentile (rudeness)
Obedience	An act or instance of obeying	Compliant, agreeable, deferential, docile (disobedient)
Fidelity/ faithfulness	Steadfast in affection or allegiance, firm in adherence to promises or in observance of duty	Authentic, believable, truthful, devoted, trustworthy (unrealistic, faithless)

Source: Peseschkian [19]. In Merriam-Webster.com. Retrieved May 8, 2019, from https://www.merriam-webster.com/dictionary/hacker

Our primary capacities to love and to know form the basis by which one negotiates life. Our primary and secondary capacities are reflections of concrete observable representations of interpersonal living. This is what Peseschkian means when he explains that our capacities and how we express them make up the contents of the unconscious. As humans, the basic capacity to love and know and the methods by which people come to know themselves, through constant self-appraisals against others close to them, are also informed by the broader culture and world around us.

A human's basic capacities to love and know underscore an infinite number of complex capacities within his/her consciousness that help negotiate existence. For example, the capacity to be self-aware or be self-appraising or the capacity to introspect and be self-reflective allows a person to develop insight into themselves. It is a complex capacity because it requires a combination of primary and secondary capacities. For example, introspection requires other capacities including time, patience, love, doubt, confidence, and honesty. Further, since many cultures incorporate past and future orientations to help them determine what value they place on different capacities, the complex capacities of *becoming and meaning making* are not static but are organic, fluid processes.

Existentialism

By 1945, existentialism was all the rage in Europe. After the fall of the Third Reich, many people living in post-World War II Europe felt that religion had failed them and looked for other ways to bring meaning into their lives. Existentialism quickly filled that void in the face of difficult social and economic times. The popular Parisian newspaper, *Le Monde*, attempted to understand existentialism but, after some consideration, could conclude a singular observation: existentialism was like faith really; it could not be studied; only lived. Davis and Miller [23] summarize existentialism from the views of Heidegger, Kierkegaard, and Nietzsche in this way:

> …there is much that is wrong with human nature. Man is an existential being whose life is more than logic and who must discover the meaning of existence. There are no answers to the human predicament to be found in the back of a book; Philosophy is to be lived, something to be proven in action… (p. 206) [23].

In much the same way, Peseschkian regarded human nature as full of possibilities. He often quoted, "regard man as a mine rich in gems of inestimable value" [24]. Positive psychotherapy demands that one "becomes yourself" which is a theme echoed in existentialism and many psychodynamic and spiritual and religious traditions.

However, many have interpreted existentialism as a philosophy that depicts human existence that is mean, sordid, and chaotic. However, that would also be ignoring the main point of existentialism entirely. Existentialism is a doctrine that affirms that every truth and every action imply both an environment and a human subjectivity [9]. Though existentialism at its core is a supremely philosophical activity, it evidences that the pursuit of purpose or life meaning is a centuries-old journey. For instance, Athenian and Roman philosophers like Socrates, Plato, Aristotle, Epicurus, Epictetus, and Marcus Aurelius searched and contemplated on a more meaningful and a fulfilling life over two millennia ago [23]. It was not just life and death that focused the attention of existentialists but themes like honesty, fidelity, accountability, and freedom evidence that the quality of life was important themes as well. See Table 39.3 for a list of common existential themes.

People do not experience life or existential themes in a vacuum. Swiss psychiatrist Ludwig Binswanger [25] reasoned that people live within overlapping domains of existence that are inextricably connected. Per Binswanger [26], these domains defined what he meant by "being-in-the-world." Specifically, this sentiment of overlapping domains of existence comprises the Umwelt (or physical dimension), the Mitwelt (or social dimension), and the Eigenwelt (or psychological

Table 39.3 Existential themes

Common existential themes	Definition	Possibilities
Life	A principle or force that is considered to underlie the distinctive quality of animate beings An organismic state characterized by capacity for metabolism, growth, reaction to stimuli, and reproduction The sequence of physical and mental experiences that make up the existence of an individual	Animation, energy, existence, being, time, living, verve
Death	A permanent cessation of all vital functions: The end of life; the state of being no longer alive	Demise, collapse, loss, bereavement, mortality, transition, end
Responsibility	Moral, legal, or mental accountability	Duty, accountability, bond, fault, task, reliability, trustworthiness
Accountability	An obligation or willingness to accept responsibility or to account for one's actions	Answerability, responsibility, liability, culpability
Freedom	The absence of necessity, coercion, or constraint in thought, choice, or action	Liberty, autonomy, independence, choice, sovereignty, candor, conformity, inhibition, self-determination
Loneliness	Being without company, not frequented by human beings, producing a feeling of bleakness or desolation	Solitude, isolation, seclusion, psychosis, apathy
Meaning	Implication of a hidden or special significance, something meant or intended	Sense, value, significance, importance, necessitating
Becoming	The consistent possibility and capability to change into something else	Attractive, charming, changing, growing, enhancing, maturing, blossoming
Courage	Mental or moral strength to venture, persevere, and withstand danger, fear, or difficulty	Bravery, nerve, perseverance, audacity, valor, cowardice
Anxiety	Apprehensive uneasiness or nervousness usually over an impending or anticipated ill, mentally distressing concern or interest	Concern, worry, nervousness, disquiet, angst, fear, reassurance, calmness
Fear	An unpleasant often strong emotion caused by anticipation or awareness of danger, profound reverence, and awe	Terror, distress, anxiety, worry, phobia, trepidation, dread

Source: Created by Marseille (2019, 2011). In Merriam-Webster.com. Retrieved May 8, 2019, from https://www.merriam-webster.com/dictionary/hacker Merriam-Webster online dictionary

or inner dimension). Not satisfied with Binswanger's three dimensions of existence, Emmy van Deurzen-Smith added a fourth dimension—the Überwelt (or spiritual dimension)—to explain the spiritual explorations of many existential philosophers, including Jaspers, Heidegger, Tillich, Buber, and Marcel. Figure 39.3 is a representation of Binswanger and van Deurzen's conceptions of domains of life that constitute *being-in-the-world.*

The *Umwelt* is the limit of an organism's world. This is true not only spatially but also in this sphere of existence: what is real is what is detectable by the senses. As Vontress points out (1979, 1996), genetic endowment and life experiences are shaped by the ecosystem in which they live. At the outset, a child's development necessitates contact with its environment. In this domain, harmony with nature in one's physical environment is not pre-established. Hence, the goal in the Umwelt is to find "one's place" or harmony with nature [28].

The *Mitwelt* is the interpersonal world. Man is a historical being, and his relationships and contacts with the world reflect history and traditions. Different groups hold different traditions valuable or sacred from others. In life, a person is molded by his collective experiences and achievements. Beyond this, he stands within the history of his own culture and family. Harmony with oth-

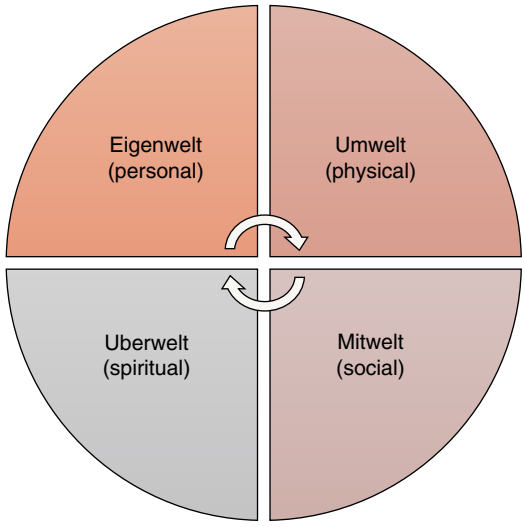


Fig. 39.3 Binswanger's "being-in-the-world" model plus Deurzen-Smith's Überwelt [25, 27]. (Graphic created by Marseille 2019)

ers is dynamic, not always given. Hence, the goal in the Mitwelt is one's ability to connect with others. It is the ability to love and be loved.

Lao Tzu [29] once said, "He who knows others is wise, he who knows himself is enlightened" (p. 117). The *Eigenwelt* is one's personal world or private realm of which others cannot experience or fully understand no matter the intimacy shared between them. Here lies one's consciousness. The Eigenwelt connotes far more than one's personal world. It is a place where complex cognitive functions take place. It attends to existential anxieties. The Eigenwelt self-appraises and uses defense mechanisms to protect it. It reminds people that they are unique. Since no one can ever fully know this part of a person, we are ultimately alone in the world. The Eigenwelt is the most personal part of one's consciousness.

Finally, the *Überwelt* is the spirit world. The spirit world connects us to those who have come and gone, to an area beyond the time-bound reality.

The search for existential insights is not novel. Philosophers around the world like Confucius, Lao Tzu, Buddha, and many others have pondered deeply about the meaning of life. Peseschkian would often quote a Chinese proverb, "When one drinks water, consider its source" [18]. He meant that every behavior has a meaning attached to it and that meaning has a cultural blueprint. Like PPT, existentialism considers deeply consequential themes about existence while recognizing one's unique attributes and capacities. Given individuals' basic capacities to love and know and the actual/secondary capacities that unpin them, one is always capable of "becoming." In other words, a person always has the capacity to change. However, the question that concerns most is: How can people change? The answer remains elusive, yet it is at the core of why people seek the help of counseling and psychotherapy professionals.

Capacity to Introspect

In *A Life Worth Living*, Mihaly Csikszentmihalyi [30] argues the human brain is unique in lieu of its ability to give rise to self-reflective consciousness. In existentialism, self-reflection, compassion, individual thought, and respect for others are central themes [31]. Existentialists view the capacity to introspect or self-reflect as an ability to think deeply about the "everydayness" of one's' existence in order to discover and reconcile their estrangement from the ordinary and mundane [12]. Schneider [32] writes, "Less and less is life animated through personal discovery, intimacy with others, or self-reflection. While life has become more manageable for many people, it has become commensurately less engaged" (p. 14). Existentialists and Buddhists share the belief that the path to a meaningful existence consists of one's capacity for compassion, self-awareness, and self-reflection and a detached analysis of one's own emotions.

In any culture time is an important primary capacity underpinning the capacity to self-reflect because people are constantly trying to create a coherent identity that requires a successful integration of the self that is based on the past, present, and future [33, 34]. A person's capacity to "become" is a significant complex capacity because it underpins all primary and secondary capacities. A person's awareness that he/she has

the capacity for change and growth, meaning making often resonates from a self-reflective process. When a person stops and asks themselves "why does my room always look a mess?" or "Why can't I ever find anything?" These questions reflect upon her capacity of orderliness, perhaps punctuality. Either way, the self-reflective process allows her an opportunity to find new meaning in some aspect of her life. Sartre wrote, "Man is nothing else but that which he makes of himself. That is the first principle of existentialism" (Sartre as quoted by Riedlinger [35], p. 28).

Capacity for Courage (Act)

The word courage comes from the Latin word *cor*, which means "heart" [36]. Kierkegaard argued that truth could ultimately emerge only from being, not from thinking, and what people lacked most was the courage to live with passion and commitment. May [37] posited that courage is the capacity to confront life "as is." Existentialists believe the capacity for courage helps individuals recognize and accept their own mortality. Courage also gives people the volition necessary to face themselves and accept who they really are [6, 38]. The capacity for courage is complex because it is a capacity that not only requires a combination of other primary and secondary capacities but also requires an act or decision. Courage helps people shun their concerns about others' opinions. People who have courage care less about fitting into ill-suited or foreign identities for the sake of insincere validation and connections.

Courage means confronting what one fears instead of avoiding it. According to Kierkegaard, courage helps people deal with the anxiety of living. In existentialism, there is a consequence for not facing life with courage. In his book, *The Concept of Anxiety*, Kierkegaard [39] argues that people tend to lose themselves when they lack courage. He explains that fear is tied to a tendency to avoid suffering and ignore their spiritual being. Because of such fear, people instead let themselves be consumed by "the world." Kierkegaard reasoned that a courageous person is prepared to suffer when he/she knows that this is required of him/her (IBID). In this view, the courage to act, to take risks to experience the "depth of existence" per Kierkegaard, may, in fact, be the only way people can develop a true sense of purpose and meaning in life. The capacity for courage to act is perhaps the most significant because it allows people to live, as existentialists would argue, "more authentic lives."

Positive Psychotherapy as an Existentialism

Positive psychotherapy as an existentialism comprises a view of existence that man is good-natured, conscious, and self-aware. Human consciousness comprises innate capacities that are influenced by experiences in the world and specifically by culture. PPT as an existentialism posits that humans have a capacity to harness life energy, which affords them the ability to *become* or change. People rely upon their basic, primary, and secondary capacities to navigate the world and to understand the complexity of human consciousness. The basic capacities of loving and knowing underscore several primary and secondary capacities that, when combined, create an infinite number of possibilities for change and growth. Together, these capacities make up the contents of the unconscious.

Meaningful Decision-Making Model

When an individual must consider a significant life experience or decision, PPT as an existentialism encourages him/her to consider the experience at hand, the emotions tied to the experience, the facts of the experience, the context of the experience (i.e., grieving someone's death, considering a promotion, moving into one's own home, deciding what car to purchase or to get married, etc.), and, finally, what to do about it. As one deploys her life energy, this logical flow of cognitions involves the engagement of various capacities and the consideration of applicable

existential themes. For example, a person engages many capacities when faced with the loss of a close relative.

The self emerges through experience [40, 41]. Binswanger argued that meaning came from a person fundamentally immersing into a world of human relating [42]. Experiences are a combination of significant and insignificant moments in time. Some experiences alter one's life, while others do not. Though experience is essential to existence, experience is unique to each person. Since people are free to choose [40, 43], they have the capacity to "surpass the given and look at things as if they could be otherwise" ([40], p. 3). Experience allows one to have this sensibility about freedom. It also allows people the freedom to "name alternatives, imagine a better state of things, [and] share with others a project of change" [40], p. 9). Getting a person to recognize that he/she is free to choose their purpose and that he/she must account for any decisions as a result of that freedom are central goals of existential counseling. Consider Fig. 39.4.

The meaningful decision-making model (MDM) is one method existentialists as well as positive psychotherapists can use to help individuals realize the weight of their own freedom and their responsibility for making meaning in their lives. The MDM model illustrates how the combination of PPT and existential concepts can work in tandem. Like the balance model, the goal of the MDM model is quite straightforward and simple. The goal of the life balance model is to help a person achieve life balance through introspection and better decision-making. Consider once more the concept of life energy. Suppose one has to make an important decision about a job. The MDM model simply argues that a person should equally distribute their life energy (25%) across the four capacities (love, know, introspect, courage) before making a significant decision (act) about any aspects of their life or, in this case, a job. Below is a basic example of how one would use the MDM model.

Jane Doe has been offered a new job but she would have to move to accept it.

- 25%: Capacity to love—How does she feel about the job? Is it a career job or just a job to help her get to a bigger career goal? Is it in her

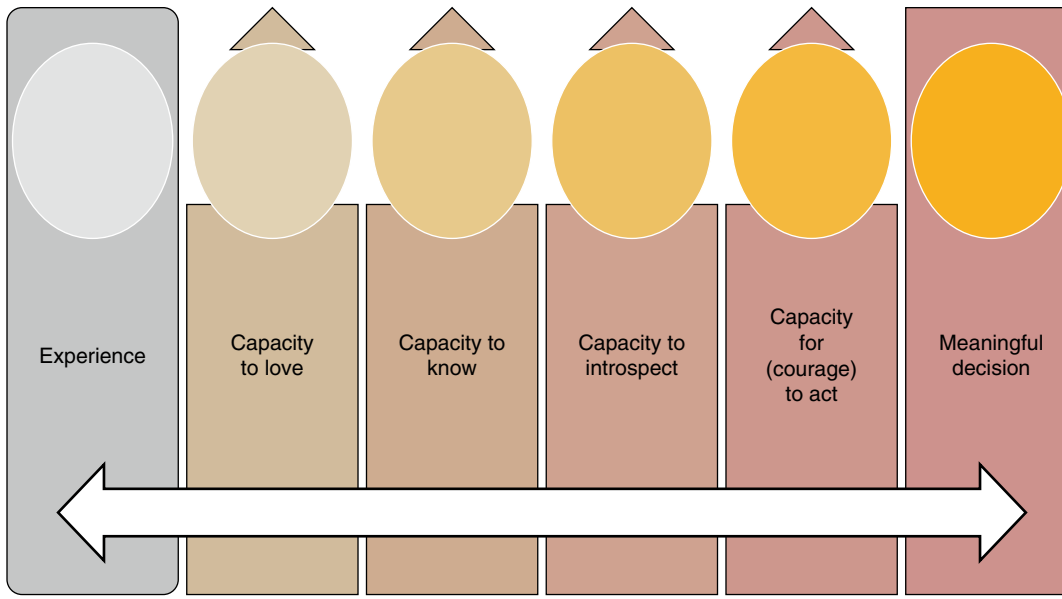


Fig. 39.4 Meaningful decision-making model. (Source: created by Marseille 2018)

field? What primary capacities are evoked when she thinks about the job?

- 25%: Capacity to act—Does she know all she can know about the job? What about the city the job is in? Is there a different climate? Is it more rural or urban? What is the diversity look like at the job? Will the benefits meet her needs? What secondary capacities are evoked when she thinks about the job?
- 25%: Capacity to introspect—Has she had similar jobs? How did she feel about those? How long does she think she will be in the job before she moves up? Is the job a fit for her long-term plans? Has she ever lived in the type of environment before? If so, what feelings does she recall having? Are there friends and family that have helpful information they could share?
- 25%: Capacity for courage—After all things are considered, can she actually make the move? How comfortable is Angela with change? What are the overall pros and cons of taking the job and moving? What are the overall pros and cons of not moving?

According to Peseschkian [44], "in good therapy, one should talk about the past 25% of the time, 25% about the present and 50% about the possibilities in the future" (p. 15). Though existential counseling emphasizes the present, using PPT as an existentialism, the therapist exploring a client's life must consider how all three dimensions of time impact his/her sense of self and worldview. Because freedom is theoretically boundless and time is inextricable from experience, the therapist must make clear that the client must understand his/her past to make any real sense of their present and plan for the future.

Conclusion

Existentialists who use psychotherapy attempt to "stay with," "stand beside," or attend to the client by exploring the client's immediate conscious experience of "being with another" or the therapist [6]. In PPT, to cultivate the client's sense of being, the therapist provides stories and words of wisdom to provide new perspectives and inspiration to the client. No matter how one disposes of the various therapeutic strategies, using existentialism within PPT helps the therapists to help clients explore deep fundamental questions of their existence, purpose, and possibilities in life through their own basic, actual, and complex capacities. Sartre wrote that the central theme of existentialism is that "existence precedes essence." He meant that a person's most important consideration is that they are "unique – independently acting and responsible, conscious beings ("essence")"—rather than what labels, roles, stereotypes, definitions, or other categories that tend to define them ("existence"). How one chooses to live their life is what constitutes "true essence" [45]. PPT as an existentialism is a means by which therapists can help clients deal with transcultural and existential issues in a world of globalization challenges and technological advances.

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40 The Development of Positive Psychotherapy in Dialogue with Other Psychotherapeutic Methods

Christian Henrichs and Gabriela Hum

Introduction

In his first book explicitly referring to his approach as "Positive Psychotherapy", Nossrat Peseschkian explained in 1977: "While many of the existing psychotherapeutic procedures take the disturbances and illnesses as their starting point, prophylactic and preventive medicine and psychotherapy require a different method of proceeding, starting from the person's developmental possibilities and capacities instead of the disturbances" [1]. Abraham Maslow, well-known for the hierarchy of needs and one of the founders of humanistic psychology, saw "the importance of focusing on the positive qualities in people, as opposed to treating them as a 'bag of symptoms." Maslow was also the first psychologist using the term "positive psychology" in 1954 [2]. Humanistic psychologists promote that every person has a latent desire to realize her or his full potential, to reach a level of "self-actualization." Overall, humanistic psychology emphasizes "the positive potential of human beings" [3]. Today, Positive Psychotherapy (PPT after Peseschkian) sees itself as a humanistic psychodynamic approach with a transcultural perspective. This pretty much summarizes its central relations and how it can be classified.

In terms of its founder's main training and its most prominent current discourses, Positive Psychotherapy is a psychodynamic modality of psychotherapy. Nossrat Peseschkian named among his personal influences his teaching analyst Heinrich Meng, a personal student of Freud, the psychoanalyst Carl Fervers with whom he did his time of clinical specialization, and Johannes Heinrich Schulz from Berlin, founder of autogenous training, with whom he took private lessons [4]. Positive Psychotherapy's "mother institute," the Wiesbaden Academy for Psychotherapy (WIAP), developed after its founder's passing into one of the leading psychodynamic institutes in Germany.

However, in the spirit of his time, the late 1960s and the early 1970s, Peseschkian was also very much part of the humanistic discourse. Furthermore, Peseschkian was—as he often stated—very much influenced by his migration from Iran to western Germany, and this reflects in the transcultural perspective of Positive Psychotherapy [4].

Peseschkian used to say in his seminars, "the human soul is good by nature; unfortunately, it is prone to alienation." This saying points to what

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_40

today is referred to as the "*positive image of man*" in Positive Psychotherapy. It might be best summarized by ideas of human nobility, of world citizenship, and of learning which are also prominent in the Bahá'í Faith, Peseschkian's religious affiliation (i.e., "That one indeed is a man who, today, dedicates himself to the service of the entire human race... The earth is but one country, and mankind its citizens," [5]). Peseschkian was born into this religious community and was a lifelong and active member. In its positive image of humans and the empathies on culture, Positive Psychotherapy seems to breathe the spirit of the Bahá'í teachings that claim to address the condition of humanity in the time of globalization.

In a broader sense, however, the idea of the human potential, of purpose, and of transformation toward goodness is shared by many of the humanistic approaches (one example being how Jacob Moreno claims that every person has their creative potential that can be unlocked through play). This humanistic perspective might contrast with the classical psychoanalytic or behavioural approaches with their more mechanistic and deterministic paradigms (such as Watson' idiom "Give me a dozen healthy infants… and I'll guarantee to take any one at random and train him to become any type of specialist I might select") [6].

Basic Assumptions

In Positive Psychotherapy, the positive image of humans is the starting point of a clear theoretical line of thought in which all concepts and models are capacity-based. This goes, as Peseschkian said in many lectures, free after Goethe who said that one should look at humans as they are and as they could be. At the core of the Positive Psychotherapy theory, there is the assumption that all of us share the *basic capacities to love and to know*. A similar assumption is made in the also Bahá'í-inspired, pedagogic "Anisa model" of Dan Jordan [6].

In Positive Psychotherapy, all the models and concepts are then derived and elaborated from these capacities to love and to know. One might relate the Positive Psychotherapy idea of capacity in the broadest sense with the psychoanalytic assumption of libido. Elaborating this comparison further, Positive Psychotherapy's basic capacity to love resembles the psychoanalytic lust principle, whereas the capacity to know resembles the reality principle. This shows both: the deep relation of Positive Psychotherapy to psychoanalysis but also its radical change in the image of human beings.

This radical change is not only of theoretical but also of practical importance as the therapist becomes a co-agent in a two-person psychology or even a multi-person one (in comparison to a one-person psychology that is represented by classical psychoanalytical and behavioural therapies). In the process of capacity development, the human aspirations of loving and knowing are acknowledged and mutually active in both patient and therapist.

Peseschkian spoke of psychotherapy in *a narrow*, *a wider*, and *a comprehensive sense* referring to the treatment of the current disorder, the development of the patient's capacity to act in her or his life, and the changing of social and cultural conditions themselves. Of course, this claim goes much beyond one-person psychology.

What we become in life is the result of three conditions as Peseschkian stated: the conditions of *body, environment,* and *time* [7]. This means both experiential and relational influences and our biological dispositions. Here, it is not so much a question of the static weight of these factors, but of how they interact in time: in a process of unfolding and reciprocal exchange—with society, culture, zeitgeist, and religion or worldview (dimensions of *giving meaning* and *finding meaning* as Peseschkian put it [7]). In the process, one's conscious self-reflection and a consultative process of social learning are important. Nossrat Peseschkian said that we are like a letter written with invisible ink. What we become in life depends on the spot where the heat is placed on the letter.

Striving for Integration

The understanding of the process of maturation of the human organism can again be linked to psychoanalysis, namely, its phases of psychosex-

ual development. Here, so to speak, also the conditions of body, environment, and time interact with the human capacities from the oral to the anal and then the oedipal phase. However, the psychoanalytic concept including the term fixation has still a quite mechanistic flavour. In Positive Psychotherapy, these classical developmental phases influence the person and are reflected in the understanding of the current interactional situation: in the *model of interpersonal interaction* [8]. Here, the stages of *attachment*, *differentiation*, and *detachment* help to describe and analyse conscious and unconscious levels of communication being shaped by transference and countertransference. Again, the psychoanalytic heritage is combined with a humanistic transformation of premises and a pragmatic approach to treatment empowering the patient as an agent of her or his positive development.

The transcultural perspective in Positive Psychotherapy does not only point to the significance of culture which is evident in Positive Psychotherapy's etiological concepts and in its interventions (e.g., *model dimensions* and *actual capacities* in its theory, *stories and proverbs* in its intervention). For Peseschkian, "transcultural" also implied interdisciplinary and integrative aspects in psychotherapy. He described the situation of psychotherapy often with the traditional oriental story of the elephant and the blind men—see Box—who only see parts of the whole. He wanted to go beyond the partial, often even ideological perspectives and to integrate. He formed the idiom: "who works alone adds, who works together multiplies."

Namely, he had great vigour to bridge the walls between psychoanalysis and cognitive-behavioural therapy. This again is reflected in his personal relationships: One of the most important co-workers in his own practice, Hans Deidenbach, was a behavioural therapist. He co-authored the strategically important *WIPPF questionnaire* (Wiesbaden Inventory for Positive Psychotherapy and Family therapy, 1988, [9]). Also, interventions such as the confrontation hierarchies regarding the actual capacities, as well as autosuggestion, or self-verbalization at certain stages of the intervention ("*psychoserum*" in Positive Psychotherapy), symptom protocols, and self-observation instructions (e.g., "*situational control*" which is similar to Ellis' ABC Technique) show the discourse with cognitive-behavioural therapy already in the early days of Positive Psychotherapy.

A last point should be mentioned in the historical perspective and Positive Psychotherapy's relations: its own learnings in its expansion to eastern countries. After the collapse of the Soviet Union, Positive Psychotherapy was very actively taught in Eastern European countries by the initiative of some dedicated trainers. One result of this is that today some of the strongest centres of Positive Psychotherapy can be found in Eastern European countries such as Ukraine, Bulgaria, Romania and Russia. At that time, the situation of psychology completely changed in Eastern Europe. Big changes also bring opportunities, and therefore, in Bulgaria, the relationship between Positive Psychotherapy and psychodrama influenced the practice of Positive Psychotherapy itself. In Germany, there was a similar mutually inspiring cooperation between Positive Psychotherapy and systemic and hypnotherapeutic teachers from Heidelberg.

The Blind Men and an Elephant

A group of blind men heard that a strange animal, called an elephant, had been brought to the town, but none of them were aware of its shape and form. Out of curiosity, they said: "We must inspect and know it by touch, of which we are capable." So, they sought it out, and when they found it, they groped about it. The first person, whose hand landed on the trunk, said: "This being is like a thick snake." For another one, whose hand reached its ear, it seemed like a kind of fan. As for another person, whose hand was upon its leg, said, the elephant was a pillar like a tree-trunk. The blind man, who placed his hand upon its side, said, the elephant was a wall.

> Another one, who felt its tail, described it as a rope. The last felt its tusk, stating the elephant was that which is hard, smooth and like a spear.
>
> (Indian traditional story, often quoted by Nossrat Peseschkian, e.g., in *Oriental Stories as Tools in Psychotherapy: The Merchant and the Parrot*, [10])

Differentiation Analysis

The publication of "*Differentiation Analysis*" historically preceded "*Positive Psychotherapy*" (1974, [11] vs. 1977, [1]). In fact, Positive Psychotherapy can be understood as an extended version of the earlier concept of Differentiation Analysis. Peseschkian explained why he chose the term "*actual capacities*, because in daily life they are, in the most diverse ways, being continually addressed at every moment." We operate in our daily life emphasizing certain (*primary* and *secondary*) actual capacities, such as *politeness* or *orderliness*—resources or even virtues—with which we got used to facing life while having other capacities left behind underdeveloped or undifferentiated. The purpose of therapy is to make the patient become aware of these capacities and to accompany the patient during the process of discovering new capacities or differentiating capacities that are too rigidly set. This describes a process of personal growth, a humanistic aspect of Positive Psychotherapy.

While psychoanalysis taught us to unmask neurosis and behaviourism taught us to debunk neurosis, Positive Psychotherapy rehumanizes psychotherapy, just like the existentialist and the humanistic therapies do. While having psychodynamic roots, Positive Psychotherapy embraces the idea that it is not practical to start from a nomic principle and to only focus therapy on identifying disorders. The innate capacities and resources of the person are equally important in the psychotherapeutic endeavour. While Sigmund Freud believed that the purpose of psychoanalysis is to make the unconscious conscious and thereby to promote liberation [12], Alfred Adler believed that the aim of individual psychology is to increase the sense of responsibility [13], and Victor Frankl believed that the aim of existential psychotherapy is to increase both the level of awareness and the sense of responsibility [14]. In Positive Psychotherapy, the intervention aims to refine and amplify the individual capacities and to perform differentiations regarding one's actual capacities (primary and secondary) [15].

In Positive Psychotherapy, another element to be considered is the past experiences of difficult situations, trying to identify the resources the patient discovered and exercised during those experiences. As Peseschkian said, "a sane person is not someone who has no problems at all, but someone who is capable of dealing with the problems as they appear" [15].

The actual capacities are central to Positive Psychotherapy's conflict model, which is biographically rooted, exhibiting the psychodynamic perspective of Positive Psychotherapy. Here, it might be noted that the fact that the actual capacities can be easily observed in everyday life (with clear operationalization, e.g., in the WIPPF, described in this book's chapter on the first interview) was elaborated much before the mainstream of psychodynamic therapy started to operationalize their concepts (for instance, in the OPD group in Germany in the early 1990s [16]). Then it can be stated that actual capacities also operate on a cognitive-behavioural level: They involve attitudes and automatic cognition as well as behavioural skills and habits.

To get a better understanding of the intriguing relevance of the actual capacities, one might reflect, for instance, on the various levels of the actual capacity of "politeness" for a moment: on its behavioural aspects as social competence, its psychodynamic ones as internalized object relationships, its socio-cultural ones as habits and practises, and the humanistic ones as values or even virtues.

The symptom, as seen by Positive Psychotherapy, is a way through which the unconscious communicates. Finding the purpose of the symptom, giving it a positive interpretation leads to the actual conflict and improves the dis-

turbed relationships of the patient to her- or himself as well as to others. It points to the capacities involved, clarifying needs, competence, but also lack and compromise. In Positive Psychotherapy, the symptom is seen as an access point to the bigger picture, not only as a technical objective as in cognitive-behavioural therapies.

Positive Psychotherapy then becomes a journey in the depths of the human being, for understanding what the person has become, as well as journey in the exterior world, to understand how the person adapted during their lifetime to the social environment and their culture of origin. This dual approach—internal and external—makes Positive Psychotherapy a unique undertaking that makes the person in front of you and her or his understanding a priority. By realizing that culture is an essential determinant in human reality and by conceptualizing its core models such as the actual capacities or the balance model, Positive Psychotherapy—nowadays often referred to as Positive and Transcultural Psychotherapy—goes beyond traditional psychodynamic and humanistic approaches [17].

While Freud recognized the emotional significance of culture (*The Uneasiness in Civilization*, 1930, [18]), it is still modelled only indirectly: In psychoanalysis, the super-ego is shaped in the interaction with the parents who confront the child with their understanding of societies' demands. In Positive Psychotherapy, the actual capacities (which might be seen partly as contents of the super-ego) are entities also in the intersubjective personal space of a cultural context involving multiple agents and different points in time.

Therapeutic Understanding

The evidence-based trend in psychotherapy (embraced especially by cognitive-behavioural schools but, more recently, also by the psychodynamic schools) challenges the different psychotherapeutic traditions. Here, Positive Psychotherapy can provide a highly acclaimed effectiveness study [19]. For this study, Nossrat Peseschkian was awarded the Richard Mertens Award for Quality Assurance, a well-known scientific recognition in Central Europe.

The Positive Psychotherapy intervention is focused on understanding the psychodynamics of the conflicts (with basic and actual conflict) that represent the foundation on which symptoms emerge. Just like in the psychodynamic schools, approaching conflicts is done by investigating the past as well as by analysing transference-countertransference situations in both therapy and real life. In other words, Positive Psychotherapy is based on a psychodynamic conflict theory. Conflicts mainly exist between the needs of primary capacities and the social norms represented in secondary capacities [3]. Also, patients are helped to develop an understanding of their initially subconscious conflict contents in terms of actual capacities.

The description of what is referred to as an actual conflict was widened by Peseschkian's *microtrauma theory*, the little events that add up to inner conflicts, in addition to the formerly researched macrotrauma theory of Holmes and Rahe life event scales [21]. Moreover, in Positive Psychotherapy, the th erapist translates the actual conflict into actual capacities which the patient has exercised less often or which are undifferentiated. Discovering the capacities involved in the actual conflict is the tip of a thread that we then follow in order to discover the intrapsychic conflicts and their dynamics. This translation makes it easier for the patient to understand since the terms used are very similar to the everyday language used by the person in therapy. Through this translation, Positive Psychotherapy brings its own contribution to the therapeutic process, making it more human and easier for the patient to further transfer it into his/her daily life. All these "translation" methods link Positive Psychotherapy again also to the humanistic approaches.

Unlike traditional psychoanalysis where the behaviour of the therapist should be abstinent and technically neutral, in Positive Psychotherapy the patient is implicitly learning from the therapeutic relationship as a model. Here, the stages of interaction are experienced, and, explicitly, the patient becomes aware of the stages that he had practiced and those that need more practice (we are talking

here about the three stages of interaction, specific to Positive Psychotherapy: attachment, differentiation, and detachment).

Positive Psychotherapy was developed as an intervention method not only for the individual but also for the family. Family therapy was elaborated around the globe during the time when Peseschkian developed Differentiation Analysis and Positive Psychotherapy. Peseschkian's idea was to combine individual therapy with family therapy [20]. This is a reason why Positive Psychotherapy is accepted in both individualistic cultures and collectivistic cultures. Positive Psychotherapy understands the symptoms in its function within the families and gives us family therapy tools for an individual member for self-help and also for the complete family in order to see the symptoms not as of one person in the family but the symptom as an expression of a conflict in the family system [20]. In comparison to other family therapy approaches, the instruments for self-help are particularly important, such as *differentiation analytical inventory* (*DAI*), *WIPPF*, *balance model*, *model dimensions*, or *five-stages procedure*.

Interventions

Nossrat Peseschkian often used humour in his therapy sessions. He used to tell his training groups that "a therapy session in which we have not laughed at least once has no result." Humour, as a mature defence mechanism, along with the use of metaphors and stimulation of creativity (sublimation) is one of the essential approaches in Positive Psychotherapy [21].

If we look at elements of effective therapy described by Frank [22], we notice how Positive Psychotherapy follows several healing principles which are universally valid:

1. *A therapeutic relationship* with emotional investment, full of trust, in which the patient is a priority. Its positive image of man holds the premise that the human being is essentially good. The therapeutic relationship is nurtured by using proverbs, stories, and accessible, respectful language. The positive interpretation strengthens the relation while challenging relational disturbances prevailing so far. As elements of attachment, Peseschkian mentioned *time*, *patience,* and *modelling*. All these elements, he claimed, are important in developing and maintaining the therapeutic relationship.
2. *An explanatory principle easy to understand* by the patient and still complex enough to allow the therapist to juggle with aspects of the unconscious life. Throughout, in Positive Psychotherapy, terms are used that are easily understood by the patient. For example, the reference to capacities such as justice, achievement, tidiness, and patience helps the patient effortlessly understand the intrapsychic and/or interpersonal conflicts and how they work.
3. *An analysis of the situation of the patient which provides her/him with the possibility to take control of her/his illness*—in Positive Psychotherapy we start from the premise that the patient is equipped with everything she or he needs both for developing symptoms and for dealing with them. Illness is a situation in which the previous problem-solving methods reach their limits, the patient being on the verge of discovering new ways of dealing with life. Thus, the illness becomes an opportunity, the symptom becomes a partner, and the therapeutic process becomes a working model for dealing with life issues.
4. *Sending out hope* with the aim of putting an end to the patient's demoralization and suffering while accepting pain and to learn to live with pain and find a meaning for it. This is especially promoted in a resourceful therapeutic relationship employing positive interpretations, stories and proverbs, verbalizing of and focussing to crucial capacities where the client lives values and/or is in a process of learning, an understandable problem analysis (actual, basic and inner conflict), and anticipating the next steps in problem solving (five stages procedure).
5. *Analysing and transmitting successful experiences* to provide the patient with more

confidence and competence. Positive Psychotherapy focuses on the patient's previous experiences in which she or he successfully dealt with similar situations and emphasizes the capacities and resources involved in these experiences (one of the key questions in the stage of situational encouragement is: what have you learnt so far from dealing with the problems).

6. *Encouraging the act of experiencing emotions as a premise for changing attitudes and behaviours:* This is done, for instance, by providing stories and proverbs that resound with the client's situation, by activating past experiences working through early family relationships with the model dimensions, by promoting inspiration and resonance by a comprehensible model such as the four areas of life, by behavioural activation employing the situational control procedure as homework, or by changing interpersonal interactions in real life situations by working on the key conflict between politeness and openness.
7. *Detachment from the symptom/problem* and experiencing it as an external element—Peseschkian used to ask the patient to write a letter to the affected organ or to represent the symptom through an object and talk to it. Through this technique, beyond connecting to one's own body, Peseschkian focuses the patient on externalizing the problem and highlighting the relationship with it.

The Five-Stage Procedure

The structured stages in Positive Psychotherapy treatment (five-stages procedure: observation/distancing, inventory, situational encouragement, verbalization, and broadening the goals) make the intervention—at least in a traditional understanding—less open than the psychoanalytic procedure where the "transferal neurosis" develops freely. However, newer psychodynamics approaches (e.g., Wöller's resource-based psychodynamic therapy for the borderline disorder [23]) also promote a more structured procedure, while Positive Psychotherapy does not come close to CBT-style manualization, but it is semi-structured. Also, the semi-structured way of organizing the patient's information in the first interview, as well as giving homework and self-reflection exercises at different stages, resembles more cognitive-behavioural approaches. The explicit focus on the objectives and future visualization from the early beginning of the therapy is promoted by questions such as "how will your life be when the symptoms are not present anymore?" link Positive Psychotherapy to short-term, solution-oriented therapy and systemic approaches in general.

In the stage of *observation/distancing*, the therapist and patient take a step back and look at the situation of the patient from another perspective. This involves the therapist's observation of the unfolding scene and its transference-countertransference (psychodynamic aspect), but it also involves a more active, empathic, and present therapist (similar to client-centred therapy). The focus is not only on the symptom but also looking at the situation in a broader sense involving strengths (capacities in terms of Positive Psychotherapy), opportunities, and cultural aspects. Here, Positive Psychotherapy relates to humanistic approaches in terms of addressing aspiration, with solution-oriented therapy in terms of aim-setting and with cognitive-behavioural therapy in terms of promoting symptom observation in everyday life. Also, the patient's understanding of the problem is observed as well as whether her or his objectives are more symptom-related or toward relationship and self-understanding. This also influences whether later practical interventions are chosen more on a cognitive-behavioural or psychodynamic level.

The *inventory stage* promotes the patient's understanding employing different models for analysis and self-reflection (WIPPF, DAI, balance model, model dimension, etc.). On the level of technical procedure, this resembles cognitive-behavioural approaches, while the reasoning itself is more a psychodynamic one. The concern of the therapist during this second stage is to find out the former experience similar with the one which the patient experiences now: the interplay

of present and past (actual conflict, basic conflict), the understanding of early complexes and objects relations (model dimensions), current decompensation (microtrauma, life events, actual capacities, stages of interaction), and current compensation (balance model). However, other resources and the patient's learning style are emphasized, too (systemic perspective).

The stage of *situational encouragement* has resource-activating aspects comparable to hypnotherapy and solution-oriented therapy. Also, strengthening exercises are applied such as relaxation techniques (behavioural aspect). Biographical longings, compensations, and learnings are appreciated (humanistic and psychodynamic aspects). The positive re-assessment of symptoms and conflicts, prepares the patient's "ego" for a more relaxed activation of conflict-related memories stored in the subconscious mind, thereby weakening resistance and promoting later reprocessing. As Peseschkian puts it, the stages 3–5 then become "anaesthesia, surgery, and aftercare."

In the *verbalization stage*, working on the key conflict—between openness and politeness—is promoted. Connected with this, inner conflicts are worked through (psychodynamic aspect), neglected communicative skills are developed (behavioural aspect), and relationships are readjusted (family or couple consultation—systemic aspect).

One of the core elements of the therapy used not only during verbalization but in all the stages, particularly in observation-distancing, is the use of stories and metaphors. The role of stories, as Peseschkian states in *The Merchant and the Parrot*, is to settle existing norms or to render certain existing norms more flexible, without directly answering the patient's questions [10]. Through this way of experiential intervention, Positive Psychotherapy refers to humanistic therapies, to the Eriksonian hypnotherapy, and—through its collective view—to the symbolistic used in Jung's analytic therapy. In another way, the use of stories might be compared to the dream analysis in classical psychoanalysis, as the well-known Austrian psychologist Peter Hofstätter stated (cited in [1]).

The *broadening of goals*, the last stage of intervention, connects the patient with potentialities of the present life situation and a creative anticipation of the future. This element links Positive Psychotherapy to existential therapy. However, more than it is done in existential therapies, in Positive Psychotherapy at the end of therapy, the three aspects of time—past, present, and future—are considered connecting the patient with her or his life in full and the process of development. Usually the patient is asked to provide feedback from the therapy and to specify the important aspects for him/her related to the therapeutic process. Also, connecting with the future is achieved by setting goals for the next 3–6 months.

Summary

Gaetano Benedetti stated in 1977 that Peseschkian's Positive Psychotherapy is a remarkable synthesis of psychodynamic and behavioural elements and thus represents an essential contribution to the unified relationships within psychotherapy (cited after [1]). From today's perspective, Positive Psychotherapy can also be seen as an early and consistent implementation of general ideas such as resource-oriented and short-term therapy. At the time of its first publication, these ideas were controversially discussed, but today they have become more and more state of the art (e.g., [12, 23, 24]). Positive Psychotherapy serves the demand "become yourself" that can be found in many humanistic and spiritual traditions. Today, the realities of life are understood to be globally influenced by more and more people. That is also a reason why Positive Psychotherapy integrates the cultural dimension into the therapeutic work and in doing so goes beyond pre-existing approaches.

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The Search: A Meaning-Centered Psychotherapy

41

Sam Cyrous

Introduction

Positive psychology states there are five elements in order to achieve a fulfilling life, in a model known as PERMA, an acronym for Positive emotions, Engagement, Relationships, Meaning and Achievement [8]. With some thirty more years of existence, another school of thought, Positive and Transcultural Psychotherapy (PPT) [4], agrees with it by saying the role of therapy is to find meaning, and our job is to accept meaning through a balance between the capacities of love and know. Logotherapy [3] offers us a room for a possible dialogue between both approaches, when it claims that meaning should be seen as unconditional, and loving life can only be real if we know that and experience that in relationships and the missions we assume in it. And only by finding meaning and living it, we can, in fact, sense achievement in life.

In English the word *meaning* encompasses two different concepts for which in Romantic languages and in German, as presented by Gottlob Frege (1892), there are two words. Meaning as *Bedeutung* (German) which is the object itself and Meaning as *Sinn* (German) which is what it intends to express. In Positive Psychotherapy, we could say, one has an idea and intends to express it by verbalizing it (produced by the body dimension) after using the creative powers (of the fantasy dimension)—this is the intended meaning (Sinn)—, while there are sounds (perceived by the body dimension of the other), with a meaning (Bedeutung) processed (by the mental/achievement dimension) of the receiver. As such only through communication can one find meaning, only through Positive Psychology's relationships or through the Positive Psychotherapy's contact dimension.

If we consider Peseschkian's thoughts, we are both indeed social and rational animals (social/contact, rational/achievement, animals/body), yet we are also moved by meaningful stories (fantasy/purpose dimension). When we feel well, we sit and tell our friends our best stories; when feeling down, we tell stories of pain and sorrow. In funerals we recall our best stories with the departed. On birthdays we mark the beginning of a new cycle, with the leading protagonists of the cycle that closes behind. When physically sick, our bodies react as if they had a story to tell (the story of a mistreated liver, or a poorly cared for stomach…); when mentally ill, we often imagine unreal situations (typical of *delusions*, *hallucinations*, and *deliria* presented in many mental disorder conditions). And when we rest, too: our brains tell us incredible stories, many times inspired by past episodes, other times recapping exactly what we have lived and, yet many more

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_41

times, stories with no appearance of sense or clear meaning. The challenge born by human beings is not accepting those stories but, rather, seeking, at every single moment, to understand their meaning.

> I once dreamt I had an interview with God.
> "Come in," God said. "So, you would like to interview Me?"
> "If you have the time," I said.
> God smiled and said: "My time is eternity and is enough to do everything; what questions do you have in mind to ask me?"
> The first question came to mind: "What surprises you most about mankind?"
> God answered:
> "That they get bored of being children, are in a rush to grow up, and then long to be children again. That they lose their health to make money and then lose their money to restore their health. That by thinking anxiously about the future, they forget the present, such that they live neither for the present nor the future. That they live as if they will never die, and they die as if they had never lived…"

Many were the questions that could be set forth by the interviewer in this dream—a story told by an anonymous Internet writer (and oftentimes attributed to the Dalai Lama). But the first and utmost one was to ask the Creator what surprised Him the most in His creation! And, better yet, the way in which the answer was built: the fact that we live in fruitless vicious cycles. We sacrifice our physical body's health to take care of professional achievements so that later we may sacrifice our victories and producing and working hours to recover physical health. The same way, we sacrifice our family relations, our friendships, the love we have towards our children to offer them what we never had, forgetting to be really there for them. And, finally, we have no glimmer of what our finitude really is, not seizing the instants life offers us because we fear life to close itself too fast before reaching what we most desire. The absurdity of all this is, nonetheless, to see the great amount of people that never find out what was, that they really desired!

And as the words of Sigmund Freud [1] echoed with a higher intensity: "We can only say '*The goal of all life is death*'". After all, how would it be if, like in José Saramago's novel *Death with Interruptions*, death would stop working? Aging, diseases, accidents and no finitude to close the doors of time and make us concerned today with the other, visit the other today, to tell the other what we feel. We would lose the opportunity to seize the moment, but also solidarity and the link that binds us together. To reflect on the meaning of our lives is to comprehend who we are and how we live a life balanced with its finitude.

Peseschkian [4] stated "strictly connected to those questions [of meaning] is the question of future". For the founder of Positive and Transcultural Psychotherapy, "time is not a formal scheme of order, but the dimension in which conflicts appear and are overcome" (2003). Thus, the role of the psychotherapist is to become a facilitator of a learning process, in which the patient deals "with their situation and, within the possibilities, [becomes capable of] finding a fit attitude towards the future" (2003).

Clinical Applications to Discussing Meaning in Psychotherapy

It is up to us, then, to understand the interlink between present, past, and future. In therapy sessions, I invite my patients to choose one of the sentences below and comment on it. After what ends up as a dialogue, I invite them to choose another sentence, this time the one they think they would have chosen, on the first session, having had access to the sentences. Their reactions are generally of astonishment and enthusiasm.

- By reading the reality, I feel my work has meaning.
- By reading the reality, I see well what motivates me.
- By reading the reality, I understand that sacrifices are worth.
- When I dream of the future, I understand many pathways.
- When I dream of the future, I feel I shall be rewarded.
- When I dream of the future, I see the result with clarity.

- When I look at life's rearview mirror, I see who I was and how I have progressed.
- When I look at life's rearview mirror, I understand I did my best.
- When I look at life's rearview mirror, I feel how much I have accomplished.

The sentences are an invitation to metaphorically think of the present (reading the reality), the past (rearview mirror), and the future (dream), at the same time as it approaches the matter of freedom and its consequent responsibility in our actions. With each choice, patients understand where they stand now, and have stood in the past. In this exercise people don't only choose their temporal focus on the present, but also revisits its past being, and perceives that, from the very beginning, they had within themselves a capacity to find the cure and/or overcome the challenges, confirming Peseschkian's claim (2003): "Healthy is not, the one who has no problems, but the person who is in conditions to solve them, overcome them"—a sentence that also confirms the Principle of Hope, a backbone for Positive Psychotherapy: patients are endowed with "capacities and potentialities (…), that enable them to find new, different and maybe better solutions (…), bringing with them not only the disease but also the ability to overcome it" [5]. As Sartre [6] would say, in its *Being and Nothingness*, the way we live makes us unavoidably responsible by the passing of time in our lives, and hence we bring our inner challenges but also the capabilities to beat them, we bring our personal wars but also the mechanisms towards peace:

> The situation is mine because it is the image of my free choice of myself, and everything that presents to me is mine in that this represents me and symbolizes me. It is not I who decides the coefficient of adversity in things and even their unpredictability by deciding myself.
> Thus there are no accidents in life; a community event which suddenly bursts forth and involves me in it does not come from the outside. If I am mobilized in a war, this war is my war; it is in my image and I deserve it.

In many ways similar to the story of Job que, as it is described in the Bahá'í version, by learning an important lesson.

> God conferred upon Job the mantle of prophethood. He was wealthy, owned a vast area of land, and lived with his wife and family in great luxury and comfort. Having been entrusted by God to guide the people to righteousness and truth, he dedicated his life to fulfilling this mission among his community. He summoned them all to the Cause of God, but they became jealous and accused him of insincerity, saying that his devotion to God was due solely to his wealth and material possessions.
> In order to manifest his truthfulness to the eyes of men, God surrounded him with tribulations. Every day a fresh calamity descended upon him. First, his sons were taken from him, all his possessions were removed and his crops burnt. Then he was taken ill and his body was afflicted with disease and covered with boils. In spite of all these calamities, he remained thankful to his Lord and patiently endured hardships with a spirit of resignation and detachment. Yet his afflictions did not end there, for he was forced out of his village with no one to help him except his wife, who believed in him and did all she could to alleviate his pain. In the end he became destitute and was without food for many days
> Job was so patient and resigned to the will of God that his thankfulness and devotion to his Lord increased with his trials. At last, having proved his detachment from earthly possessions, God again bestowed upon Job all that was taken from him. His teachings spread and his words penetrated into the hearts of the sincere, enabling them to recognize and acknowledge his station (cited by Taherzadeh 2001) [10].

Once told, this story may cause mixed reflections. In some cases, people get even disturbed with the story; in one specific case, after hearing it, the person was in such a revolt that would simply refuse, in his own words, to accept such an aggressive god, who would punish those he claimed to love. Which takes me to what is now my first question every time I tell this one story:

- *Does the story suit only religious people?*

When asked and brought to clarity, people understand that by taking it off the religious field and putting it in their personal reality, adults of both gender and young people from the most diverse aging groups, they may question themselves on their actions, and their own responsibility towards the processes in their lives and, even, it there would be a moment, a place, a situation in

which they would not be responsible for their choices and actions. They come to realize that it is not about being in the presence of a cruel or testing god but rather, in front of the changes and chances in life, that we cannot and shall not control, but that may look as if they control us. And when we take our focus off the problem, we may foresee the many elements that may help us to solve it. This is why, I invite my patients to think in the functions of the story, as Peseschkian [5] has taught us, with the following questions:

- *What are the capacities you see in the main character?*
- *What other stories from the same genre do you know?*
- *Do you consider this to be a real or fictional story?*

The image of having a being in unjust suffering allows those who listen to the story to more easily identify with the theme of the story, reflecting about their own lives (mirror function), being able to test new response models to be applied to the conflicts that brought them to the session (modeling function). The main point here is to provoke in them to consider the following question:

- *What are the stories of suffering that we live and how to face them?*

Peseschkian [4] reminds us how "the subject is on a conflict between two possibilities that possess their light and shadow" or, in the poetic words of Oscar Wilde [11], set forth in the screams of Dorian Gray: "Each of us has Heaven and Hell in him"). It is from this metaphoric revelation of constant trekking between heaven and hell that is made possible finding meaning to our own existence.

As such, we should always bear in mind our answers to questions such as the following two:

- *What makes each of us unique?*
- *What binds us together?*

For some finding our uniqueness is a challenge, while for others our togetherness is an endeavor. Perhaps meaning in life is like the forces in physics; one may have to empower centripetal or centrifugal forces, going away from togetherness to know and love oneself, or abandoning self-centered vision of things in order to embrace more things. Yet, we may never forget the idea of balance often presented by Peseschkian. So, refraining, perhaps our main task as therapists is to assist people to find this balance between their (sometimes strong, sometimes forgotten) centripetal or centrifugal forces, between themselves and others. Just like what Buber proposes in I-Thou, such a person is truly free, and as such, is both self-aware and able to sacrifice themselves.

A First Person Perspective on the Diversity

Sometime ago, someone told me diversity is dangerous for all human dimensions. Dwelling between so many different cultures—my genetic origin is Persian, I was born Uruguayan, and raised in Brazil, while my youth years were in Portuguese land—I couldn't even dream of agreeing. But an outsider may tell me I am lost, I am so unique there's no possible togetherness for me, there is no Buberian Thou or Peseschkianian Contact or even a Seligmanian Relationship; there's only room for uniqueness. This lays down the question: *are we united in potential or is our diversity stronger*? This question teased me for many years, up to the day I started noticing and appreciating the typical Portuguese pavement stones. Each one externally limited, sometimes ugly, and with imperfect edges. Aren't we, humans, also like this, each of us imperfect in its own way? Beauty rises when we put these stones together and create pavements with the most beautiful images, only visible to those who see the whole picture and not only the single stones. In the same way, "If all men were perfect, then every individual would be replaceable by anyone else", like mosaics and stained glass perfectly cut and outlined seen in gothic German churches (Frankl [2]). Just as death, as a temporal limit, is a constituting element for meaning, our imperfections are limitations and thus assist us in find-

ing our meaning. So, by being unique and consequently different and diverse, our diversity implies a unique and unrepeatable individuality, while, at the same time this is what makes the mosaic so beautiful.

Individuals and Community as Co-creators of Meaning

> Viktor Frankl, with whom Peseschkian told me in electronic mailing on August 07th 2009 "spent a part of my psychotherapeutic education with", and with whom "have worked together with (…) for a while in Vienna and elsewhere", in August 12th, says: The significance of such individuality, the meaning of human personality, is, however, always related to community. For just as the uniqueness of the tessera (tile) is a value only in relation to the whole of the mosaic, so the uniqueness of the human personality finds its meaning entirely in its role in an integral whole. Thus the meaning of the human person as a personality points beyond its own limits, towards community; in being directed towards community the meaning of individual transcends itself (Frankl [2]).

We are talking about a community that enables the development of the full potential of both the person and the collective, in a sort of double relationship with society and personal development, where one may find the meaning of their life. It is as if because of our limits that we are enabled to develop individually: our physical limits are the ones we try to surpass, the psychological conditionings that we try to overcome and the social rules that we try to change. We are capable of evolving, individually and collectively, because of such limits. Limits are what make life meaningful!

Peseschkian asserts that we all possess two basic capacities: to love and to know. The first provides the emotional bond to give and receive love, and the other the rational bond to learn and teach. Perhaps a third basic capacity is imperative for our scope—the capacity to seek, free and independently, the meaning of our existence: a pre-rational and pre-emotional, innate capacity unique to our species, a capacity some may try to ignore (because of fear, ignorance, unreadiness, lack of reflection). The alienation of this process of seeking for existential meaning is what, many times, tosses us towards depressive or anguished symptomatology and even suicidal though or action.

PPT's Balance Model and Logotherapy's Search for Meaning

Seeking to understand our own limits, so we may, afterwards, love and know them can be transformational. If we think within the sphere of PPT's Balance Model, where we have four dimensions that ought to be balanced, we may realize how some patients arrive at the sessions and how the therapeutic process should be. This precept rooted in Positive Psychotherapy tells us we have the *physical* dimension and the physical body ruled by natural laws; a *mental or achievement* dimension that allows concrete and solid goals (often turned towards careers we now call professional); a *socio-emotional/relationship* dimension where traditions and family, friends and other relations are; and, lastly, a *transcendental, spiritual, intuitive* dimension turned into the realm of dreams, fantasy and the future. This is the point that unites all human beings. The way in which we seek to live each one of these limits is what takes us to find a meaning in our lives or, in the words of Peseschkian [4]: "Despite all cultural and social differences and the peculiarities of each person, we observe that all people resort to typical ways of dealing with conflicts so they may overcome their problems"—we are different in what points out to the essence, but united in overcoming problems and conflicts.

Peseschkian [4] teaches us we need "first" to make "a global differentiation of what is me and not-me", being thus the role of the therapist to assist people in consultation to find their limits between me and not-me in all the dimensions.

When we are turned towards the **physical dimension**, our bodies react to the conflicts—headaches, pain in many organs and muscles, psychosomatization of many issues, difficulties in sleeping, in sexuality or in eating. We find out an alleged ugly image of ourselves (being it adulthood, weight, wrinkles or aging as a whole),

struggling with it or accepting it. We may be Dorian Gray whose picture-mirror becomes uglier and older by the day while we protect, at every cost, the body, or a Stephen Hawking who accepts his physical limitations to overcome them, leaving his legacy as one of the highest geniuses of contemporaneity. I constantly invite patients to think of their bodies—*is this the body you want?, how did you come to discover it?*, *what are the physical impairments?*, *do you like seeing yourself in the mirror?*, *do you appreciate food, eat too fast/slow?*. Some of these questions, in our post-modern society where the body has become an empty temple, cause pain, since most are not aware or even know how to deal with the limits of their bodies, or even respect other people's bodies (assuming they own the other, sometimes through physical and sexual violence). It is thus mandatory to educate the generations of youth and pre-youth to know their bodies and accept the diversities they put forth, despite their similarities.

When focused on the **achievement dimension**, we confuse work with our own being, and any criticism is held as personal; and, to not hear them, we stick our heads in work or run away from the critics and work all together. We live expectations that are hard to achieve, with fear of failing, of assuming who we are, of coming out of the programmed, of the alternatives, of ridicule. Questions are made on the *indecision before fear*, *other people's opinions on personal abilities*, *competitiveness*, *consternation towards the ridicule* to better understand the way we live this dimension and its limits. Once, during therapy process, a patient, a Brazilian public high-school teacher, in his first session, told me he was tired of feeling like Sisyphus, always pushing the useless rock mountain up, to which I offer a contrast asking why doesn't he try, rather, to be Prometheus who also had a mission to accomplish, and risked eternal damnation for infracting the norms and, eventually, becomes the true hero for humanity since he was the one who brought the flame of knowledge to people. In the many sessions to come, the patient claimed to increasingly be Prometheus, doing his best for his students: to transcend the limits of achievement dimension that offered him conflict to achieve his meaning of sharing knowledge to others.

In the **socio-emotional/contact dimension**, our focus is on relationships. When conflict arises here, we become dependent on others or too independent, because we don't know how to connect. Feelings may be in constant boiling, or totally withdrawn because of obvious fears. It is important to ask how they live their limits with and within their families, their couple life, their friends.

To whom do you express your emotions?

Do you have any difficulties expressing them?

How do you feel before the disappointment of not being who you would like to be? / the other would like you to be? / before finding out the other is not who you thought them to be?.

In this dimension we find the dichotomy Romeo and Juliet, by the one hand, and Majnún e Laylí, by the other: both cases show the absence of the other causing an apparent separation, but in the first one separation took as to a meaningless suffering in which both die and, consequently, it is implies they don't end up together, and, in the second, both are graved side by side, implying that in the after-life there would be a reencounter. By reading both narratives, we learn we all have feelings, but what differentiates us is if we are capable of living in this dimension's conflict, without seeing its meaning, or, instead, we find meaning in the limits of suffering that emotions may cause.

And, last, the **future, spiritual, fantasy, dream dimension**, perhaps the dimension that makes us truly humans. In an allegorical sense, it would be the dimension of the alpinist who scales and tracks a mountain; he sees only the clouds ahead, not the summit and may give up; another alpinist understands in the thin air and strong wind the proximity to the peak and doesn't abandon the journey. It is here we have the ability of interpreting reality; in the previous dimensions we may see, work, like reality or not, but here we are capable of interpreting reality, and we may even be wrong! But this is the dimension of the "in spite of": in spite of physical conditioning, in spite of critics and fears, in spite of our feelings we make the difference, change the ecosystem—

it is the dimension of the unlimited, of possibilities and, consequently of responsibility for what I do and/or decide not to do. Deeper conflicts oscillate here between connecting or not to our essence, accepting who we are or fighting against it, accepting a life of freedom without responsibility (indulgence? whim?) or one of responsibility without freedom of choice (and consequently guilt). It is perhaps the dimension "in the core of our existence, in the disposition to serve all things, good and bad, beauty and ugly, living and dead", which Scheler [9] calls humbleness.

One may find this balance in popular stories of yore, such as the *Conference of Birds* (by 'Attar), or modern-day versions of tv-shows such as *Star Trek* with peoples of different species. In both cases one may find a bird, or an alien, more focused on achievement, or a character more focused on a bodily and practical perspective of life. Only by going together can they achieve their missions and grasp the prize it unfolds them, may it be the Phoenix in one case, or peace and prosperity in the galaxy in the other.

It is from the balance between the limits the four dimensions present us that we are enabled to seek the meaning of our lives. It is like a beautiful garden. Our physical dimension allows us to plant the flowers, with achievements we organize the location of each one considering their needs of sun and nutrients, with contacts we enable those that need solitude or presence be comfortable in the whole and, lastly, with future dimension, we visualize the garden months before, knowing our efforts shall be rewarded. The meaning of the garden isn't in any of the dimensions, it is in the four of them together. The meaning of the garden is not in the possibility of doing anything anyways, but rather, knowing the limits of what, when and how to do. The meaning of the garden is not in the present nor in the past, but in the capacity to deal today with the learnings of the past, to create a multicolored future.

And this is perhaps the ethics professed by Sartre [7] who said:

> And, when we say that man is responsible for himself, we do not mean that he is responsible only for his own individuality, but that he is responsible for all men. (…) When we say that man chooses himself, we do mean that every one of us must choose himself; but by that we also mean that in choosing for himself he chooses for all men. (…) To choose between this or that is at the same time to affirm the value of that which is chosen; for we are unable ever to choose the worse. What we choose is always the better; and nothing can be better for us unless it is better for all.

What may we understand here? That (1) our happiness is interlinked to the happiness of the whole and, furthermore, (2) ethics is to choose, free and responsibly, something we all, in equal circumstances would choose. The sense of seeking a meaning for our lives is only possible if we respect this rule. Our responsibility is with humankind as a whole. In the words of Mandela it is in the relationship with the other that we find a meaning: "What counts in life is not the mere fact that we have lived. It is what difference we have made to the lives of others that will determine the significance of the life we lead".

A true challenge for the clinical therapist is to seek which of the dimensions is mostly developed, and try to work through a balance. People are embedded with all four, but not always they are visible to themselves. A useful technique may be telling a set of stories, in what can be called StoryTherapy. The receiver of the story usually assimilates the power of stories, changing the way they sense its meaning, and hence, they change the way they sense meaning in their lives. While we tend to perceive things (body) and rationalize them (achievement) and only then, perhaps have it clear how we feel about them (contact), stories change all the order. Thus, after perceiving information (body), we reflect upon it (fantasy), we find and/or develop feelings (contact), and then we may rationalize (achievement) in order to complete the cycle by doing something (body). It is, again, about balance between the four dimensions.

And yet, when there is no balance, but inner conflicts between dimensions, there's some level of meaning alienation (fantasy), and dysfunctional feelings (contact), thoughts (achievement), and actions (body).

Limits as Doors to Meaning

We need to understand and accept limits, or conditions if one prefers. Time, death, the environment, rules of nature or law, and even our bodies are limited. But limits are not walls one cannot go through. Limits are like doors or gates.

A gate represents the limits between (a) what we choose and (b) what we decide not to choose, between (a) the a choice itself and (b) the proactivity to open or not to open it Gates are the limits of a life that has just been born since the door to this world has been opened, and the limits before death which closes the door of life. They are the tresholds that separate feelings such as sorrow and happiness on the other side, or the border that imbues us with fear for what is on the other side. In the European middle ages, an X on a door would represent that within it there was a plague, in Andalucia (Spain) there are moments and cities where the doors are opened so tourists may see the beauty of inner gardens. For the Hindus, it is important to decorate homes with *Bandanwar*, put over the doors, to attract and please Lakshmi, the goddess of wealth. In Islamic tradition one may find the *Jannah*, the paradise temple with its eight doors, each describing the spiritual practices and virtues a true believer must have. John (10:9) recalls Jesus calling Himself the *thŭrā* (greek for door or entrance), through which one must enter in order to "be saved". In this same line, Bahá'ís call one of their two Founders *Báb*, which in Arabic means the door or the gate through which comes a new era that unites and separates present, past and future of Humankind.

Your door is the first thing seen by your guests (by the Buberian Thou that is in front of I). Doors limit our space. The eyes of someone turned to the physical dimension see them as structures (and ask about their material, their thickness and size, their color, and even about their beauty), understanding them all as equal in some aspects and diverse in others. The eyes of those turned towards the achievement dimension perceive that some serve better their security functions then others, assess the easiness to open them without the key, the easiness to copy the key, the fears of something going wrong and the assurances to go right, they perceive the strategic strengths and weaknesses of the doors. Those who look at the doors focused on contact think of uniting those who are inside the door's premises and separate those who are out of it; delimiting those who are together in a group from those who are not from the group. It is always from this limit between *what is* and *what is not* that we come to grasp *what may be*! And when we unite these dimensions to the fantasy dimension… we learn that the door needs to be opened, but in the correct way; if we hold a heavy weight, open it to the inside may be hard; if many people want to come in, open it to the inside may bring new people, ideas and information inside, without prior outing of who/what should leave; if stuck, pulling it will only increase the difficulty of opening it, since we need to push it to open. This is why, fantasy dimension is so relevant. It is this last dimension that makes it possible to comprehend that the door must be opened from within out, allowing our openness to the diverse, the different, the external and the new, so that *I* and *Thou* find meaning, and hence happiness together.

In the words Frankl has attributed to Kierkegaard: the door of happiness opens only from inside out. Or, after Peseschkian, the door of happiness opens if we know its mechanisms, love what we may find out there and, eventually, we seek to open it.

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Positive Interpretation as a Tool in Psychotherapy

42

Solomon Abebe Woldemariam

Introduction

Thought has a mighty power to govern human routine activities. What we are showing today is a replica from our thoughts of yesterday and will shape our life tomorrow. This is indicated in one of the Buddha's sayings: "If a man speaks or acts with an impure mind, suffering will follow him as the wheel of the cart follows the beast that draws the cart. If a man speaks or acts with a pure mind, joy follows him as his own shadow" (Buddha, as cited in [6]). This clearly demonstrates the idea that thoughts and emotions can affect mental health status. Negative emotions, on the other hand, can have damaging effects on mental energy, physical health, achievement, social relationships, and spirituality and potentially lead to serious health issues.

Mahatma Gandhi resonates similar ideas: "A man is but the product of his thoughts; what he thinks, he becomes" [9, 10]. Thought manifests the visible from the invisible and it is crucial to recognize that what we think, say, and do returns to us. If individuals work on the capacity to change their thoughts, they can develop the capacity to change their world [22].

Aristotle also said "the energy of the mind is the essence of life" (as cited in [17]). For Aristotle, rational thought was the highest form of human nature and can guide both moral and intellectual virtues. Aristotle emphasized that living involves using reason to govern emotions and desires. He added that thought acted as the foundation of human flourishing. Thus, for Aristotle, nurturing a disciplined and virtuous mind is essential for gaining happiness and well-being.

On the other hand, Sigmund Freud, the father of psychoanalysis, contributed a different dimension to the understanding of thought. He proposed that much of human behavior is governed by unconscious thoughts and desires, rather than purely conscious reasoning. He further proposed that unconscious thoughts—often repressed memories, desires, and fears—significantly influences emotions and behavior [7, 8]. This interaction between the conscious and unconscious mind is vital to understanding how thoughts affect mental health.

A clinical research conducted among psychotherapy clients ($N = 140$), in Ethiopia, showed the following common irrational thoughts or thinking errors: all-or-nothing thinking, overgeneralizing, filtering out the positive, mind-reading, catastrophizing, emotional reasoning, labeling, fortune-telling, and personalization [1]. After the therapist supported clients to make positive interpretation, most clients explained that positive interpretation will create real value in their life and supported them to identify and build skills that last much longer than a smile. Most of the

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_42

respondents affirmed that positive interpretation helped them develop other life skills and gave them an opportunity to enhance a sense of possibilities. Research respondents mentioned that positive interpretation helped them reduce routine stress and improved their level of confidence. A significant number of respondents mentioned that positive interpretation gave them insight to develop new skills and resources for self-healing. Several respondents described that the new approach of seeing positive capacity behind challenges empowered them to create a peaceful and balanced life principle across their body, achievement, social contact, and spirituality.

Clients' experiences in childhood, social norms, and cultural influence might affect the capacity for interpreting problems positively. Therefore, lack of positive interpretation might be the generating factor for worsening mental health, suffering from different psychosocial problems—a process where our conscious and subconscious mind play a great role.

The conscious mind is responsible for filtering and examining thoughts coming from outside and it passes filtered messages to the subconscious mind and what conscious mind believes to be true, the subconscious mind will accept and bring to pass [14]. On the other hand, the subconscious mind will have a responsibility to implement thoughts directly received from our conscious mind and it doesn't weigh the pros and cons of the thoughts. If it receives success, it will serve as a catalyst to implement success and if it receives failure it just facilitates to implement failure.

Generally, the human mind plays a seeding method. Therefore, if we plant potatoes, potatoes come out and if we plant tomatoes, tomatoes come out and if we plant apples, apples come out. However, it is impossible to expect potatoes after planting tomatoes. By the same analogy, if we plant positive thoughts positive thoughts will come out and if we plant negative thoughts negative thoughts will come out. Every one's life is in his hands, to make of it what he chooses [12] and positive interpretation can serve as movers and shakers on one's life to create a stable mental strength: managing thoughts, regulating emotions, and behaving productively despite circumstances. When people change their irrational beliefs to undogmatic flexible preferences, they become less disturbed (Albert 2019).

Definition of Key Terms

Thoughts

A strong psychodynamic definition of thought comes from the psychoanalytic perspective, where thought is understood as both conscious and unconscious mental processes that influence behavior, emotions, and decision-making. In this view, thoughts are often shaped by underlying, unconscious desires, conflicts, and past experiences that are repressed but still exert influence over the individual's conscious mind. According to Freud [7], thought is a product of the interaction between the id, ego, and superego. Therefore, thought in the psychodynamic/psychoanalytic sense is not merely conscious but deeply affected by unconscious forces. Often, to bring underlying thoughts to conscious reflection, it requires psychodynamic/psychoanalytic intervention.

Positive Interpretation: Comprehensive Overview

Positive interpretation is a technique used in Positive Psychotherapy after Peseschkian. It works by re-interpreting problems and avoiding passive attitudes thus reflecting the ability of clients. With this interpretation, the therapist does not solely address the illness alone, but also the subjective ideas about the illness as transmitted through the traditions of the family. In line with this, depression may be understood beyond the feeling of being sad, or irritable, with predominantly passive attitude and instead reflect the ability to react with deep emotionalism [15]. Another way to re-interpret depression is by looking at it as a result of depletion of the available energy during the period prior to the onset of the episode and that the depressed state represents a kind of hibernation, during which the

patient gradually builds up a new store of energy (Aron 1967).

Irrational Thoughts or Cognitive Distortions

Irrational thoughts are unrealistic and generally negative thinking patterns and individuals might fall into and obsess over. These thoughts can be infuriating, discouraging, and in some cases downright hazardous. Thinking errors contribute a lion's share for irrational thoughts. The most common thinking errors or cognitive distortions are: all-or-nothing thinking, overgeneralizing, filtering out the positive, mind-reading, catastrophizing, emotional reasoning, labeling, and fortune-telling [4].

Applications

Positive Interpretation is a core concept of the Positive Psychotherapy (PPT after Peseschkian). Positive Interpretation should be considered across all five stages of PPT consultation: distancing/observation, taking inventory, situational encouragement, verbalization, and broadening of goals. To bring effective positive interpretation, the therapist should be curious on the following points: Understanding the place of positive interpretation in the client's cultural context, supporting the client to define positive interpretation, identifying individual and social factors influencing positive interpretation, and identifying barriers to positive interpretation.

1. *Understanding the place of positive interpretation in the client's cultural context*: In Positive Psychotherapy understanding the place of positive interpretation within a client's cultural context is important for effective therapeutic practice. Positive interpretation allows a nuanced approach that respects and integrates the client's cultural values and beliefs. Furthermore, positive interpretation comprises framing situations in a way that highlights strengths and opportunities, which can vary significantly across different cultural backgrounds. As Hofstede [11] explained, in individualistic cultures positive interpretation might focus on personal achievements and self-reliance. However, in collectivist cultures, positive interpretation may emphasize community and family support. After considering the client's cultural context, therapists should modify interventions that resonate specifically with the client's worldview. Therefore, integrating giving due attention for cultural perspectives during positive interpretation increases therapeutic efficacy and client satisfaction [21].
2. *Support the client to define positive interpretation*: From a psychodynamic perspective, supporting a client in defining positive interpretation involves exploring unconscious processes and early life experiences that shape their current thought patterns and emotional responses. According to Freud's theory, unconscious conflicts, particularly those rooted in early relationships and experiences, can contribute to negative or maladaptive interpretations of events [7]. By bringing these unconscious influences into conscious awareness through techniques like free association and dream analysis, clients can gain insight into the origins of their negative interpretations. This awareness allows them to reframe their perspectives, promoting healthier, more positive interpretations of their experiences [3]. Additionally, psychodynamic therapy emphasizes the therapeutic relationship as a means of helping clients experience a corrective emotional experience, which can further support positive re-interpretations of their past and present challenges [13].
3. *Identify individual and social factors influencing positive interpretation*: Individual and social factors significantly influence positive interpretation, shaping how people perceive and respond to their experiences. On an individual level, personality traits such as optimism, resilience, and self-efficacy play a key role in fostering positive interpretations of events (Bandura 1997). People with a higher sense of self-efficacy are more likely to view

challenges as opportunities rather than threats, promoting a positive outlook. Social factors, including cultural background, family dynamics, and peer influences, also contribute to how individuals interpret situations. Collectivist cultures, for example, may encourage individuals to view challenges through a communal lens, emphasizing shared support and collaboration, while individualistic cultures may foster interpretations that emphasize personal growth and achievement [11]. Social support networks further enhance positive interpretation by providing emotional and practical resources that help individuals reframe stressful situations more constructively [5].

4. *Identify barriers to positive interpretation*: In the psychodynamic approach, barriers to positive interpretation are often rooted in unconscious conflicts, unresolved childhood experiences, and defense mechanisms that distort perceptions of reality. According to Freud's theory, these unconscious conflicts may lead individuals to develop maladaptive thought patterns or defense mechanisms such as denial, repression, and projection, which act as barriers to interpreting events in a positive light [7]. For instance, an individual who experienced early attachment issues may unconsciously interpret current relationships through a lens of mistrust or fear, making it difficult to see positive aspects in interpersonal interactions. These deep-seated unconscious fears and anxieties create internal resistance to positive interpretation, often manifesting as automatic negative thoughts. Furthermore, psychodynamic theory suggests that transference—when clients project past feelings onto present relationships—can obstruct positive interpretation by coloring current experiences with unresolved emotions from the past [13]. Therefore, addressing these barriers in therapy involves helping clients uncover and process these unconscious dynamics, enabling them to reinterpret their experiences more positively and adaptively.

In Positive Psychotherapy, the therapist's role is to act as a facilitator rather than an advisor, avoiding directing, judging, or giving advice to the client. This non-directive approach empowers the client to take an active role in their own healing process, promoting self-discovery and personal autonomy [15, 16]. The therapist's primary function is to help the client reframe their problems in a positive light, facilitating alternative perspectives and fostering resilience [19]. Each session should be approached critically, ensuring the process of positive interpretation is maintained and strengthened. Regular follow-ups are vital to prevent relapse into negative thought patterns and to help clients continually reinterpret their challenges constructively [15, 16]. Positive reinterpretations serve as stimuli for rethinking ingrained beliefs, promoting the consideration of new forms of treatment and solutions [20]. This long-term approach to therapy not only helps in immediate problem-solving but also fosters personal growth and well-being by encouraging clients to reassess their situations and make positive changes in their lives (Joseph and Linley 2006).

In Positive Psychotherapy, therapists must address key barriers that prevent clients from adopting positive interpretation, which can be categorized into personal, social, and environmental obstacles. Personal barriers often include a lack of information, low perception of risks, poor understanding of benefits, and a lack of skills or confidence needed for positive reframing [15]. Social barriers encompass peer influences, prevailing social norms, and societal expectations of what is considered normal behavior, all of which can hinder positive interpretation. Environmental barriers include poverty, restrictive laws, inadequate mental health policies, and a lack of access to psychosocial services, all of which can limit a client's ability to engage in positive reinterpretation [15]. The therapist's primary goal is to help clients overcome these barriers by fostering alternative attitudes toward their challenges and encouraging therapists to adopt a flexible, adaptive approach to treatment. This holistic perspective empowers clients to shift their understanding of their circumstances and fosters resilience and growth.

Traditional interpretation	Positive interpretation
Depression	
The feeling of being despondent, with a prevailing passive attitude. Spiritual dejection, exhaustion	The ability to react to conflicts with deep emotions
Phobia	
Fear of certain objects, such as mice, dogs, spiders, or of specific situations	The ability to avoid situations and objects which are experienced as threatening
Bedwetting	
Bedwetting is a special form of uncontrolled emptying of the bladder. It occurs at night, when one is lying in bed asleep.	The ability to react to conflicts with deep emotion
Paranoia	
Madness, insanity, mental illness with primary crazy notions (delusions of persecution, grandeur, reference, among others)	The ability to see oneself as the midpoint of the world and its secret power.

Some positive interpretations are capable of being enlarged up on primary capacities, secondary capacities, body achievements, social contact, futurity, and model dimensions (I, you, we, primal we).

Clinical Vignette

The Ability to React to Conflicts with Deep Emotion

Rebuilding Life Through Positive Interpretation: A Case Study of a Client's Journey from Depression to Balance

The client is a 48-year-old engineer. He is the father of a son and a daughter. He is in the process of divorce when one of his construction sites burned; in addition, he lost a significant amount of money and changed his religion two times. Primarily, he was diagnosed with depression in Addis Ababa, Ethiopia. After 2 weeks of psychiatric care in the hospital, the client referred to follow-up and positive psychotherapy service.

During the first interview the client observed with the following irrational thoughts or thinking errors: persistent sadness, anxious or empty mood, feelings of hopelessness, pessimism, feelings of guilt, worthlessness, helplessness, loss of interest or pleasure in hobbies and activities, including sex, decreased energy, fatigue, and feeling slowed down. He was even reluctant to receive positive psychotherapy. However, he was expecting appropriate service to eliminate his irrational thought and thinking errors, confusion and return to his normal life and reconcile with his wife, restart his previous business, and get rid of his superstitious thought. Initially, the therapist requested the client to positively interpret his personal problem. He replied that depression is depression and it is a lack of interest in everything that could be. Then, the therapist supported the client in interpreting his problem as the ability to react to conflicts with deep emotion and critically observed and checked client' change across the five-stage consultation. The process also enlarged up on his model dimension, including dimensions of time, trust, and hope. During consultation he realized that his life components are unbalanced and decided to give due attention to his body, futurity, and social contact (See Chapter 8 on the Balance Model). During the termination phase, the client explained that positive interpretation created a real value in his life and helped him identify and build skills that "last much longer than a smile". In addition, the positive interpretation supported the client being able to see other life skills and gain an opportunity to increase his sense of possibilities. Moreover, he mentioned that positive interpretation supported him to diminish routine stress and boosted his confidence and ignited his mind to develop new skills and resources. Because of this positive interpretation he was able to create a peaceful and balanced life principle across body, achievement, social contact, and spirituality—the Balance Model.

Summary

Positive interpretation plays a crucial role in addressing clients' irrational thought distortions or thinking errors. In Positive Psychotherapy

(PPT), as introduced by Peseschkian, clients enter the therapeutic space not only with problems but also with potential solutions. In this framework, the relationship between the therapist and client is one of equality, where the client is not viewed merely as a patient, but as an active participant in the healing process. This collaborative relationship creates an environment conducive to helping clients reinterpret their problems positively [15, 16]. Positive interpretation is shaped by a combination of personal, social, and environmental factors, and may require time to produce meaningful and impactful results. It serves as a tool for clients to identify and replace irrational thoughts such as all-or-nothing thinking, overgeneralization, filtering out the positive, mind-reading, catastrophizing, emotional reasoning, labeling, and fortune-telling [2]. Setbacks or relapses are common during the five-stage consultation phase, and the ability to change negative self-talk during these setbacks is a key component of optimism [18]. Therefore, it is essential for the therapist to monitor and evaluate the progress of positive interpretation critically in each session.

Key Points

We should give priorities to the following points during positive interpretation in Positive Psychotherapy (PPT after Peseschkian).

- Be informed that the patient is entering the therapy room not only with his problem but also with potential solutions.
- Understand the client's experiences and cultural values towards positive interpretation.
- Differentiate client's personal barriers, social barriers, and environmental barriers to positive interpretation.
- Support the client to interpret his problems positively and gear his interpretation with ability or capacity. Then, positive interpretation will create a fertile ground for clients to eliminate his irrational thoughts or thinking errors.
- Relapse or setbacks are very common among clients. Therefore, the therapist should remind and support his clients to reinterpret his problem across the five stages of consultation.

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Part VI

Emerging Topics and Contemporary Issues

Humor in Positive Psychotherapy and Psychiatry

43

Karina Melo Saboya and Filipe Saboya Santos

Introduction—The Setup

Does everybody have a sense of humor? Does humor have an essential function? In the game of evolution would humor be a fitness play off? In today's stress-filled, overworked, sleep-deprived world, is there a place for humor? How can it be used in clinical practice for positive health? Can a sense of humor be developed and improved? These topics are the focus of discussion in this chapter.

The 2024 World Happiness Report reveals that there has been a decline in happiness among the youth in recent years, particularly in North America, where it has fallen sharply. These data correlate with those found in the "Cliff of Humor", a study of 1.4 million survey respondents in 166 countries in a Gallup poll revealed that "we're all going over the humor cliff together, tumbling down into the abyss of solemnity below. The frequency in which we laugh or smile each day starts to plummet around age 23, when we enter the workforce and trade laughter for ties and pantsuits, as we become serious and important people" [1]. The average four-year-old homo sapiens laughs as many as three hundred times per day. The average forty-year-old laughs three hundred times every two and a half months [25]. Paul McGhee, a notable researcher of the origins of sense of humor, addresses that some adults suffering from "terminal seriousness" have lost touch with a fundamental aspect of our nature—a biological predisposition to play. In order to thrive, humankind must find well-being amidst civilization and its discontents. A sense of humor stands as a powerful and essential tool [20, 21].

The *Homo Ridens* is being muzzled by the civilizing and educational process [2]. The behavior of having a belly laugh, much like experiencing ecstasy, appears to be a behavior at risk of extinction. As the civilizational process advances, laughter and humor are gradually set aside, being removed from churches and schools, and never truly made their way into medical offices.

In the book "The Name of the Rose", the Italian Humberto Eco uses the historical context in the Middle Ages to show the Catholic Church had an ambiguous view of laughter. Aristotle's fictional and poisoned book in the novel reveals the fear of the subversion contained in comedy and laughter. The fictional book on comedy is a reference to the real "Poetics" where Aristotle discussed tragedy, but left the section on comedy incomplete or maybe it has not reached our days. The same can be observed in the painting "The Wedding Dance"

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_43

By the Dutch artist Pieter Bruegel. He captured a scene that was considered disrespectful in 1536: swinging legs and arms, dancing, and laughing loudly. Taking time away from patients' suffering to write about jokes, puns, riddles, and playfulness was frowned upon and might not always seem like a serious thing to do.

The term "humor" originates from the health sciences of Hippocrates, the Latin word "humor" means fluid or moisture. According to ancient, medieval, and Renaissance physiology, people's illnesses and temperaments were classified according to their bodily humors—blood, phlegm, yellow bile, and black bile—into sanguine, phlegmatic, choleric, and melancholic. The balance of the humors determines whether a person is in a good or bad mood. This theory persisted for over 2000 years, from the fourth century BC until the seventeenth century. People who were thought to have an excess of one of the four bodily humors became subjects of mockery and were called "humorists". This term soon evolved to describe anyone who tended to create amusing, absurd, or ridiculous situations and ideas. By the late 1600s, the word "humor" began to shift in meaning, becoming associated with things that are funny [20, 21]. Whether it's due to the pleasure humor brings us or the substantial amount of money comedy generates, this sense has predominated ever since.

Today, cognitive scientists continue to classify people in relation to their humor; each individual has or practices humor in their own way. This is known as a sense of humor. Sense of humor is the way humor is appreciated and practiced by an individual, their habits or styles of humor that differentiate them from others [28].

Humor, scientific insight, art, and other forms of creativity are all based on a mental process with characteristics that can be called "bissociation" and divergent thinking. The structure of a joke typically follows a basic framework, which can be broken down into the following key components: setup, turn, and punchline. In the case of humor, bissociation causes an abrupt transfer in the train of thought from one matrix to another, governed by a different logic or rule of the game [18].

Positive Health—The Turn

Are we healthy? The positive revolution or the revolution of levity is also a turn in the health sciences and a response to the need to understand and practice what makes humans the healthiest they can be. It directs the focus toward the strengths we have, rather than what we lack, or what we have, but shouldn't have, like dysfunctional symptoms. It has a perspective oriented toward the high performances of athletes, artists, and extraordinary individuals. It addresses personal development, emotional intelligence, and sustainable connections with the universe around us. Virtues, positive emotions, flow experiences, and overall flourishing are all regarded as valuable health assets and central themes in positive psychology and psychiatry.

A special technique of Positive Psychotherapy (PPT after Peseschkian) is the therapeutic use of tales, stories, proverbs, and humor. "Humor is like salt in the soup of therapy" (N. Peseschkian, quoted in: Messias et al. [19]). Humor is a valuable asset found both in cultures and individuals, and it is something that can and should be cultivated and strengthened. Many great thinkers widely support this view. "Humor is by far the most significant activity of the human brain" [9] is a conclusion achieved by Edward de Bono, a physician, psychologist, and pioneer in the field of creative thinking and problem-solving. His methods and ideas, like lateral and divergent thinking, have had a lasting impact on all fields that involve creativity and better decision-making processes [10]. "Humor: Serious Business" is an MBA course at Stanford University by psychologists and writers Jennifer Aaker and Naomi Bagdonas. "Laughter is good because it makes it easy to admit new ideas", thinks Dalai Lama, while Freud considers humor a higher form of intelligence [1].

It is important to bring these great and wise people into the discussion, before we proceed with the chapter, as the subject we are addressing has for centuries not been considered a serious matter, often left in the hands, body, and minds of the Court Jester. Although the Joker's role may have seemed frivolous, with his whims and witti-

cisms about the hypocrisies and contradictions of courtly life, he as the only one authorized to make criticism, was in fact crucial in maintaining equilibrium in the king's life and reign, keeping him grounded in his humanity and preventing him from being swept away by excessive power.

Nowadays, those who have best managed to harness the potential power of humor so far, seem to be the entertainment industry, which every day further refines its tools and methods to offer us laughter in exchange for our money. Meanwhile, scientific and medical interest has not moved forward as quickly. Although there are indeed some good examples of this scientific interest as we will see next.

Clinical Applications for Positive Health

The therapeutic benefits of humor are becoming gradually more evident, with more research showing how humor can alleviate symptoms of stress, anxiety, and depression. The activation of the brain's reward circuitry through humor suggests potential applications in neuropsychiatric treatments, especially in enhancing patient engagement and adherence to therapeutic interventions. And perhaps, hopefully, it may lead to drug development that could target specific brain areas or neurotransmitters more specifically involved in a positive sense of humor.

Emerging research suggests that humor might play a role in enhancing cognitive flexibility and promoting neuroplasticity. This is particularly interesting in the context of aging and neurodegenerative diseases, where maintaining cognitive resilience is crucial. The ability to understand and appreciate humor might also serve as a marker of cognitive health, offering a potential diagnostic tool for early detection of cognitive decline [26].

There's also exciting work being done at the intersection of neuroscience and artificial intelligence, particularly in how machines can be trained to recognize and respond to humor. This has implications for developing more sophisticated brain-machine interfaces that could potentially improve communication for individuals with neurological impairments.

Developing or improving a sense of humor can have profound benefits for both mental and physical health. Programs designed to enhance humor skills can help children, adolescents, and adults tap into the therapeutic and social power of humor and laughter, leading to greater resilience, improved relationships, and overall well-being [16].

As follows below, humor is discussed in the context of the four aspects of the Positive Psycotherapy's Balance Model: body, relationships, work, and spirituality, the four areas of our lives that we should keep in a playful rhythm [24].

Body and Health

Recent research has provided us with many insights into how humor operates in the brain, which might have significant implications for both clinical practice and our understanding of human cognition. The cognitive model for explaining humor is used in almost all other areas of humor study, which is why we emphasize it here the incongruity-resolution model [15].

> "Is the doctor home?" asked the young patient, whispering in a hoarse voice as he arrived at the doctor's house.
> "No," whispered back the doctor's young and beautiful wife. "Come in quickly!"

A joke takes our brain through a three-stage path that leads us from ambiguity and confusion to pleasure. The brain's function to understand a joke, as evidenced by detecting brain activity using brain MRI, is the same area that is activated when we have an insight. The three stages our brain is guided by the joke, constructing, reckoning, and resolving, are key not only for humor but also for all aspects of complex thinking. We are, by nature, hypothesis-generating creatures.

The hypothesis constructed in the first stage of that joke is that the patient is going to the doctor to seek a cure. We have then a conflict of ideas: the doctor isn't home, and the patient is invited in. The second stage, Reckoning, is the process of reevaluating these misperceptions, usually lead-

ing to a pleasant surprise. The resolving problem is that they are lovers! The joke is more interesting as more distant the two oppositional ideas are: the set up and the punchline—the seek for cure and the seek for sex.

In the constructing stage, we produce many hypotheses, most of them wrong and we generate conflicts, with many neural circuits activated at the same time and managed by the anterior cingulate, which silences the wrong answers, allowing the desirable answer or possibility to appear, activating the reward circuit and producing dopamine.

In humor, discovering a misinterpretation leads to laughter through a mechanism of reconciliation with the complex, conflicting, and incongruent nature of our brain and the world. Marvin Minsky, scientist pioneer in the artificial intelligence field, in his TED Talk "Health and the Human Mind," explains that we have approximately 400 brain areas, many of which we do not fully understand, but we know that different processes are constantly occurring, and they don't always work in harmony. He agrees with Freud's theory that these processes often contradict and cancel each other out, revealing, from a neuroscientific perspective, our inherently conflicted and ambivalent nature. The punchline of a joke serves as a resolution, often nonsensical, that brings us pleasure [32]. As illustrated by the following example:

> *The patient says, "Doctor, last night I made a Freudian slip. I was having dinner with my mother-in-law and meant to say, 'Could you please pass the butter?' But instead, I said, "You foolish cow, you've completely ruined my life".*

The use of fMRI and other neuroimaging tools has allowed us to observe humor processing in real-time, revealing the intricate network of brain regions involved. The right prefrontal cortex's role, particularly in detecting incongruities and resolving them (a key component of many jokes), has been reaffirmed, while the reward system's engagement, particularly the release of dopamine in the mesolimbic pathway, underscores humor's role in emotional regulation and social bonding.

Humor still maintains its connection to physiology, the balance of neurotransmitters produced by the body, such as oxytocin and cortisol and also between sympathetic and parasympathetic system control [27]. A good joke followed by a hearty laugh releases a cocktail of hormones that make us feel happier (dopamine), more trusting (oxytocin), less stressed (lowered cortisol) and even slightly euphoric (endorphins), more or less like meditating, having sex and having a runner high [1]. When you laugh, there's an initial activation of the sympathetic nervous system, often referred to as the "stress response", where your heart rate might briefly increase. However, this is quickly followed by the more lasting response of the parasympathetic, the "rest and digest system" which helps to bring the body back to a state of calm. This transition can result in a decrease in heart rate, lower blood pressure, and a general feeling of relaxation.

By reducing stress through parasympathetic activation, humor can positively impact immune function. Chronic stress can suppress the immune system, making the body more vulnerable to illness. When the parasympathetic system is engaged through humor, the body shifts into a state that supports healing and immune function.

Children and adults who are more engaged in enjoyable and fun activities have lower systemic inflammation, as indicated by lower levels of pro-inflammatory cytokines like interleukin-6 and C-reactive protein. Patients with chronic obstructive pulmonary disease experienced improved lung function after being entertained by a clown. In a fifteen-year longitudinal study, encompassing over fifty thousand individuals, Norwegian researchers unveiled a correlation between sense of humor and longevity. As it's often said "having fun and being fun might just be the secret to feeling alive", it appears that this secret also holds the key to staying alive longer [1].

An important implication for clinical practice relates to sleep problems. Since the parasympathetic system promotes relaxation and reduces stress, engaging in humor before bed might help improve sleep quality.

In a study we conducted to validate an Scale of Humor Quociente we found a positive relationship between sense of humor and sleep quality, as evidenced by our findings in the Positive Affect Test, the Sense of Humor Scale, and the

subscales Discovering Humor in Everyday Life, Laughing at Yourself, and Humor Under Stress, which confirms divergent external validity with Sleep Disturbance—Short Form Adult, Patient-Reported Outcomes Measurement Information System (PROMIS) [8, 29].

This relationship between sleep and humor is a promising field of research and is certainly worth referencing. Neuroscience showed [31] that after 24 hours of sleep deprivation, the frontal lobe—where much of the humor processing occurs—shows a decrease in glucose utilization, indicating lower functioning. In a study [17] on humor perception using a combination of cartoons and verbal humor, it was found that neither caffeine nor other stimulants like modafinil or dextroamphetamine improved humor detection performance, although they did enhance performance on attention tasks that did not involve humor.

The benefits of sleep for memory consolidation are already well-known, as is the effect of humor on learning. In another study [7], researchers tested the "humor effect" on memory consolidation after sleep and confirmed that lack of sleep negates the "humor effect." That is, humorous content, which was more easily memorized than non-humorous content, loses this advantage when sleep is lacking [15]. It's worth noting that the correlations in our study occurred with the overall score and with the last three subscales of the Sense of Humor Scale which represent the most difficult humor-related skills to develop, according to the program theory..

To conclude: Less sleep, less humor! Humor helps our body relax and fosters a natural inclination to connect with others to enjoy more humor.

Relationships and Contacts

Laughter and humor are inherent to the human condition, a mechanism that allows people to connect more deeply and empathetically, recognizing their own flaws and those of others. According to psychiatrist Viktor Frankl, who survived concentration camps, humor does not serve to deny pain, but rather to allow a more human relationship with it — and with others. In groups, it acts as a social lubricant, protecting the self without harming the other, and fostering more authentic connections. The will for humor is a useful trick for the will to live [12].

Using humor is an art that can be learned. Negative humor styles, such as aggressive humor—especially when there is excessive sarcasm or humor inappropriate for the situation—should be strongly avoided. On the other hand, adopting positive styles, like affiliative and self-enhancing humor, enhances emotional and social competence. Learning to apply these styles in an adaptive way is akin to knowing how to dose a medication correctly.

Humor theory dictates that, for humor to occur, three roles are required: the joke-teller, the listener, and the subject of the jest. Quoting Bergson from his book "Laughter":

> The logic of what is funny is the logic of a dream, but it's never just an individual fantasy; it represents what an entire society dreams of. What is comical primarily elicits laughter because of a social function. 'Laughter conceals a tacit understanding, I'd even say almost a complicity with other laughing souls, be they real or imagined... our laughter is always that of a group... it's not a mere abstract play of ideas that the mind revels in) (Bergson [5], p. 32).

Humor is essentially a social ability. An ability to be used around others. Recent studies have delved into how humor is processed differently depending on social and cultural contexts. The cognitive understanding is expanded by McGraw's Benign Violation Theory to include emotional and moral aspects [22]. This has implications for understanding cross-cultural communication and the role of humor in social cohesion. The work on mirror neurons highlights how humor might enhance social bonding by allowing individuals to share emotional experiences, even in complex or ambiguous social situations.

We begin laughing around the third month of life to facilitate the peaceful differentiation between mother and baby and continue to use laughter throughout life to resolve conflicts and strengthen relationships.

The smile, particularly in early childhood, serves a vital purpose: it captivates caregivers, ensuring the prolonged attention and care needed

during the extended dependency of a human child. This initial smile, a simple yet profound connection to others, evolves in complexity over time, eventually developing into a sense of humor. Yet, the smile remains most powerful and fulfilling when it retains its primal function—as a means of connection and bonding with others.

In 2015, the "crying with laughter" emoji was chosen as the Word of the Year by the Oxford Dictionary. Unfortunately, Monalisa, the emoji, and the increasingly frequent phrases like "wow, that's funny" often lack a genuine smile. Humor is the most effective and faster way to evoke this kind of laughter when needed.

French psychologist Guillaume Duchenne using electrical stimulation of facial muscles and photography discovered and reported:

> The emotion of genuine joy is exposed on the face by the combined contraction of the zygomatic major muscle and the orbicularis oculi muscle. The first obeys the will, but the second only comes into play through the sweet emotions of the soul [4], p. 33).

In a more recent study using image technology, the lips of Monalisa, the lady "Gioconda" ("the smiling one" in Italian) was divided in half and mirrored each half, resulting in two pictures of two different smiles. Due to the resulting smile's lack of symmetry and according to the impression of various respondents, it appears that Monalisa doesn't have a genuine or a Duchenne smile.

Laughter is among the most highly contagious affective expressions [14]. The saying "Smile and the world smiles with you" is a piece of wisdom that has long been part of popular culture. In science, the famous study where volunteers held a pen between their teeth, forcing a smile, demonstrated that smiling not only influences others but also affects our own brain, fostering reciprocity in the world and within our own emotions. The muscles activated when we smile send feedback signals to the brain, indicating an experience of joy, even if the external circumstances or personal situations haven't produced that particular emotion—in other words, even if the smile is forced [20, 21].

As Freitas-Magalhães, a Portuguese psychologist, teaches us in his book "Human Smile" smiling is both an innate and learned behavior. Humans can lose instincts when they are not stimulated, and laughter cognitively evolves into humor.

The smile holds a biological survival value in the early years of life [13]. It is both an innate and learned behavior because, like animals, humans lose instincts when they are not stimulated, which can happen with the phenomenon of smiling. The smile may be innate, but it is dependent on social interaction.

Some research suggests that facial expressions are necessary for an emotion to be fully experienced; if the facial expression does not occur, the emotion may not be felt [13]. Although not all studies confirm this finding, the emergence of laughter yoga in India and its proliferation around the world seems to be practical evidence of this concept.

Work and Achievement

> ***"The supreme accomplishment is to blur the line between work and play" Arthur Toynbee*** (British historian who examined the rise and fall of civilizations throughout history).

A touch of comedy can offer significant benefits even before someone is hired. In a study, MBA students were paired to simulate job interviews and negotiate employment benefits via email, including aspects such as salary, bonuses, and insurance. Each benefit was scored to determine whether recruiters or candidates secured better advantages. Interestingly, students who began their messages with a Dilbert cartoon—a character known for its satirical take on the corporate world—scored higher. Moreover, their peers perceived them as more trustworthy and reported greater satisfaction with the process [1].

The workplace is inherently stressful, the decline in laughter, highlighted by the Gallup poll, is often observed during the weekdays. Our era presents a significant challenge: we are organic beings who need sleep, rest, and vacations. Yet we are constantly subjected to rapid, unrelenting change, driven by technological enti-

ties that never rest, and evolve at a pace difficult to keep up without increasing stress levels.

Fear derails creativity, whereas levity guides it to the right track. Humor can create a sense of psychological safety, enhancing resilience, and interpersonal connections. The key is to strike a perfect balance between levity and gravity, just like the hot and cold combination in a hot fudge sundae.

Humor and laughter can assist us in the process of relearning to cooperate. As posted by the philosopher Allan de Botton: humor is still waiting to be discovered as a tool for building a better society [3].

Spirituality and Meaning

"*Humor é uma forma de reza*" (Humor is a form of pray), Chico Anysio (Brazilian comedian).

This quote reflects the perspective on the positive and uplifting nature of humor, suggesting that it can bring joy and relief to people in a way that some find in prayer or spirituality.

Transcendence is one of the virtues cataloged in Positive psychology "Values in Action Inventory" (It is available on the internet if you want to find out which are your most powerful strengths). https://www.viacharacter.org/survey/account/register (see Fig. 43.1).

Humor and spirituality are strengths within the category of transcendence, among the personality strengths that forge connections with the universe, with something beyond us, rising above a situation and altering one's perspective.

An enriching perspective to explore when adding meaning to experiences through humor is the Joker's view, interpreted through the lens of Eastern philosophy, articulated by Alan Watts. The Joker perceives life as a game. However, this is not just any game—it's a profound one. The idea of the game, basically, is this: the nature of the world is musical. The world is doing all these forms of trees and stars and people and all their complexities just to do them. It has no purpose beyond doing it. It is exactly the same way, in music: music has no destination… Music itself is dance. And likewise, in the art of dancing, you are not traveling, you are not aiming at a particu-



Fig. 43.1 Virtues and strengths in positive psychology

lar place. You are dancing to dance. And so, what you might call the musical or game theory of the world is that everything that is happening is its own point [33].

To change perspective is the mind movement of humor, from a knowledge and directed mindset to a playful one. Comedy helps us deal with the complexity and absurdity of life [21].

Positive Intervention

The influence humor has on these four areas of our lives, body, relationships, work, and spirituality, might be underestimated, and as a result, we are missing out on its benefits. I want to share something Daniel Barros, a Brazilian psychiatrist, mentioned in his book "Rir é Preciso" (which translates to "Laughing is a Must," though it hasn't been translated yet). He said: "You are not using your sense of humor to its full potential. It's frustrating to hear. It's like that strange feeling we get when we discover, after many years of using something—an appliance, software, an app—that we haven't been using it correctly" [4]. A sense of humor is an innate app we all possess; we just need to learn how to use it effectively.

Many thinkers already recognize that to achieve our life goals, we would do better to focus on our habit systems rather than the goals themselves. Medicine in general, and lifestyle medicine in particular, increasingly emphasize the importance of cultivating good habits to achieve health. The development of a sense of humor is also based on habits! French philosopher and Nobel laureate, Henri Bergson, celebrated for his insights into time, consciousness, and his seminal essay "The Laughter", believed the relationship of ideas that become funny, follows a particular logic that resembles the logic of dreams or the collective unconscious**:**

> To reconstruct the logic of the comic**,** a very particular effort will be required**,** by which the outer layer of well-defined judgments and firmly established ideas will be suspended. Images will then emerge that interconnect not by chance but following laws or, above all, habits that are to the imagination what logic is to thought(Bergson [5], p. 54).

He is talking about habits of imagination! Who could imagine that our imagination, which we presumed to be so free, is slaved by habits?

Minois (2003) [23], in his treatise on the "History of Laughter and Ridicule", asserts that the history of laughter is nothing more than the history of the evolution of mentalities. Carol Dweck's work on mindsets teaches us "The mind is a world of its own, capable of turning heaven into hell and hell into heaven and change happens not through revolution but rather through habit"[11]. Amy Cuddy's study on power postures also highlights the transformative power of consistent habits. Her mantra is "Don't fake it until you make it. Fake it until you become it" [6]. We are habit creatures. Recent studies highlight the significant impact of consistent training on skill acquisition.

Positive health goes beyond the absence of disease; it encompasses a sense of flourishing that is nurtured by three interconnected roots: meaningful social connection, cultivation of positive emotions, and ongoing personal growth. The emotions tied to humor, its creative thought processes, and the development of the habit of using it can play a key role in this positive health revolution.

We have been applying a program "Hábitos Para-Simpáticos – Estilos de Humor" (Parasympathetic Habits – Humor Styles) to practice humor habits with adolescents in Fortaleza, Ceará, Brazil. The goal of the program is to integrate humor into the four areas of the "Balance Model" of Positive Psychotherapy. This intervention is based on the steps outlined in the "7 Humor Habits Program", the pioneer humor intervention, crafted by American psychologist Paul McGhee, who stresses that humor is a skill you can enhance in 14 weeks. His program has been tested at Zurich University with some promising results [28]. It is assessed through two pre-tests for the foundations of sense of humor and the Sense of Humor Quotient – HQ, which we validated to Portuguese in a study at the University Fernando Pessoa in Porto, Portugal [29]

Conclusion—The Punchline

To conclude, just as in positive psychotherapy, small doses of art and culture can work wonders:

The parable of the Poisoned Arrow

Once upon a time, there was a man who had been struck by a poisoned arrow. Instead of seeking immediate help, he obsessively sought answers: Who shot the arrow? What was it made of? The archer's age? The type of feathers used? While he pondered these questions, trying to understand, the poison spread, eventually claiming his life.

This parable highlights a desire for understanding that eclipses the experience itself, the will to live. To truly grasp the nature of humor, one might need both intuition and direct experience. Despite ongoing study, researchers have yet to reach a consensus on what exactly makes something funny.

Words from Lenny Bruce, the famous creator of modern stand-up comedy:

> I'm not a comedian. The world is sick, and I'm the doctor. I'm a surgeon with a scalpel for false values. I don't have a show, I just talk, and I have a sense of humor.

The American comedian faced numerous legal battles due to the content of his performances, leading to a series of arrests and court cases. His struggles against censorship had a significant impact on freedom of expression in the United States. After his death in 1966, Bruce was gradually recognized as one of the most important and influential comedians of the twentieth century. In 2003, he was posthumously pardoned by the governor of New York for his obscenity convictions, a gesture that symbolizes the shift in attitudes toward freedom of speech and the role of comedy in society.

Positive health is measured along a dynamic cognitive-emotional-social spectrum. Through this positive approach and the use of humor, patients—or more accurately, all of us—can navigate challenges with greater grace and ease.

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44 Climate Crisis and Mental Health: Applying Positive Psychotherapy to Navigate Psychological Risks

Meike Pudlatz

Background

> Climate change is the greatest health threat to humanity (WHO, 2021) [1]
>
> It is past time to focus attention on the far reaching toll climate change is already taking on our mental health (APA, 2021) [2]
>
> Climate Crisis is a psychological crisis – whatever else it is (Poulsen, 2018) [3]

Climate crisis is not really an appropriate expression for the ongoing situation. The word crisis is associated with difficult and unpleasant circumstances that are going to pass in one way or another. But we are heading toward a global catastrophe, which is very hard to realize. 2023 broke a lot of sad records in global temperature, ocean heat, sea level rise, etc. and it does not seem to change [4]. Nevertheless, we need to do whatever we can to keep the impact as small as possible. As humans, we are connected to the climate crisis and all related problems in three ways: we are the ones causing it, the ones suffering from it and the ones responsible for solving it. The given psychological constitution and the extent to which we are affected have a great impact on how we deal with these three aspects. The following considerations are written from the perspective of a western industrialized country, Germany.

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As healthcare professionals, we are primarily concerned with the people who suffer. At the same time, we also have a social responsibility to contribute to healthy living conditions. In Germany, for example, this is laid down in the professional code for psychotherapists [5]. That means that we strive to both cure and prevent (mental) suffering. Thinking of prevention there is a huge variety of possible actions we can take. For example, we can help inform people about the health risks of inadequate climate protection and thus strengthen push factors for changing climate-damaging behavior. On the other hand, we can also serve pull factors by emphasizing the health benefits of climate-friendly behavior, e.g., in the sense of the planetary health diet. Many behaviors that benefit the climate also benefit health [6]. This approach is called the co-benefit approach. In Germany, it is mainly advocated by healthcare professionals [7]. They recommend behavior such as increased cycling and walking, a diet with fewer animal products and more social interaction, i.e., through sharing goods. This is relevant for mental health as well.

With regard to the climate crisis, we find ourselves in several roles at the same time. We have to take care of ourselves and our social environment, e.g., family and friends. We bear responsibility for our patients, who belong to a vulnerable group within the climate crisis [8]. And finally we (should) play a specific role as members of a professional group in society. To find a healthy

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_44

balance within these different responsibilities is quite a challenge. In order to minimize suffering from climate change and build up climate resilience we should engage in mitigation, adaptation and transformation both on an individual and collective level [9]. As many people in society place a lot of trust in doctors and psychotherapists, we can make a very meaningful contribution in order to maintain healthy living conditions.

After these general preliminary remarks, the chapter will focus on two main themes:

– What health risks to the psyche are associated with the climate crisis.
– How can Positive Psychotherapy and other methods contribute to solutions.

Mental Health Risks Due to the Climate Crisis

Mental stress is caused by direct and indirect effects of the climate crisis and is heavily dependent on the individual's state of health and the availability of resources, resilience factors and options for action. To address these circumstances can be a first step to reduce mental burdens. The less people feel powerless, the better it is for facing the climate-related challenges in a healthy way—the reverse also being true.

It is important to note that increased, sometimes intense, emotions of fear, anger, or sadness in the context of the climate crisis are appropriate emotional reactions to a real existential threat. Climate anxiety is a realistic fear and therefore not a disorder requiring treatment. However, the intensity or frequency of climate-related emotions can overwhelm the psychological defense mechanisms and cause or worsen mental disorders. So-called climate emotions therefore can function as triggering factors [10, 11].

Before going into more detail about psychological stress, let us briefly recall the direct and indirect consequences of climate change [12].

The direct effects of high greenhouse gas emissions are: It is getting hotter, in some places stormier, heavy rainfall events take place more often, droughts are increasing, weather patterns remain stable for longer, ocean temperature is rising, ice and permafrost soils are thawing, ocean and wind currents are changing...

The consequences of this include: drinking water shortages, increasing wildfires, crop failures and food shortages, destruction caused by floods and storm surges, rising sea levels, decreasing habitats for humans and animals, an increase in zoonoses, loss of biodiversity, new disease burdens…

These circumstances lead to: people losing their houses and loved ones, "climate anxiety" and other climate emotions arise, flight and migration increase, conflicts and wars over resources can arise, and the need for change is increasingly recognized.

How Climate Change Affects the Psyche

We can differentiate three ways of how mental stress is caused by the climate crisis:

- Immediate changes.
- Longer-term changes.
- Knowledge about the climate crisis and its consequences.

Immediate stress is caused by extreme weather events such as storms, floods, and wildfires. These can lead to panic, despair, acute stress reactions, and subsequently to PTSD, depression, anxiety disorders, or addiction [13, 14]. Many healthcare systems, even in Germany, are not ready to appropriately help many people affected by those events. Immediate stress is also caused by periods of persisting heat. The impact of heat on body and soul are widely underestimated. It causes concentration problems, aggression and suicidal tendencies increase. Heat can also worsen physical illnesses and lead to problems caused by inappropriately dosed medication. For example, tricyclic antidepressants reduce sweating, so a change in medication is recommended in case of persisting heat [15]. Especially very young and elderly people as well as people with

severe mental disorders like psychosis are affected. They are often handicapped in carrying out self-care behavior like drinking enough or shadowing themselves. Therefore, German healthcare institutions recommend therapists to inform their patients about risks and appropriate behavior [16].

Long-term changes can lead to psychological distress due to the need for adaptation and behavioral change. Increasing temperatures or exposure to allergens can change working patterns. People may have to work remotely or at different times of the day, which affects social life as well. Flood risks can force people to move or leave their familiar surroundings. Social vulnerability and effects of gentrification should be seen here as well [17]. In general, the reduced planning security and existential insecurity pose a psychological challenge. Previously successful coping strategies and lifestyles are losing their functionality. People get more vulnerable and easily irritated. Furthermore, many people find it difficult to change their habits in order to reduce greenhouse gas emissions, in Germany especially with regard to car mobility, meat consumption, and the general consumption of resources. The German lifestyle would require three Earths if copied by everyone. Finally, the psychological consequences of other diseases that are spreading or worsening, such as prolonged allergies or an increased risk of zoonoses like COVID-19, are also among the longer-term burdens.

Last but not least, *knowledge of the climate crisis* and its effects is not without consequences. An increase in worries and fears can be observed, particularly among young people and their families. Hickman et al. surveyed 10,000 young people, aged 16–25 years, in ten countries and found across all countries that 84% were at least moderately worried about climate change (59% were very or extremely worried). Four out of ten young adults hesitate to have children because of the climate crisis [18]. Another international study with 130.000 participants showed that 89% demand intensified political action and 69% are even willing to spend 1% of their income on climate action [19]. The new term *solastalgia*, coined by the Australian environmental philosopher Albrecht in 2005, impressively describes the increasing grief over irretrievably destroyed nature, the loss of comforting, emotionally significant places such as forests, lakes or villages [20]. Especially among older people (in industrialized countries), the knowledge of the man-made climate crisis also leads to feelings of guilt and shame, which are sometimes dysfunctionally warded off [21]. The energy needed to suppress knowing what's going on increases. Time pressure for change is high, which is perceived strongly by parts of the population. At the same time there are other parts of the population that deny or reject the pressure to change as inappropriate. Additionally, some political procedures are not made for organizing quick changes. This leads to anger, conflict, and social tensions, in some cases to radicalization and the formation of "bubbles". Social cohesion is in danger. There are more and more people unwilling to talk to those who think differently. Trust and contact within close relationships or families can become highly irritated by that and intergenerational conflicts occur. Young people might accuse their older relatives for not taking enough action or care too little about their future. Children and teenagers also suffer from a lack of role models or advice and orientation from their parents or teachers in this situation. Family dynamics can become very conflicting. Looking at people who take action toward fighting the climate crisis we often observe that they reduce other activities in order to have time for their activism. For them it is difficult to establish a good balance between self-care and activism. Activist burnout occurs [22]. On the other hand, it can be observed that more and more people are withdrawing into their private lives and avoiding news consumption. According to a German study, only 39% watch international news on a regular basis. Asked about their expectations for the future, there is a huge 50% difference between private life (73% optimistic) and life in general (23% optimistic). It seems like people keep their optimism by reducing their horizon [23]. Thus, it becomes more difficult to make joint social efforts to improve the situation.

Positive Psychotherapy Can Help to Counter the Climate Crisis and Its Consequences for Mental Health and Increase Resilience

Positive Psychotherapy (PPT after Peseschkian) is used worldwide in various fields, including coaching, counseling, and education in addition to psychotherapy. The following considerations can be transferred—with slight modifications—to the various fields of application and are therefore not limited to the psychotherapeutic setting. Being aware of the climate crisis and using PPT methods can also have an effect in the private sphere (as a role model), the social sphere (education, exercise) as well as in professional education (workshops, lectures). In therapeutic work, we have to deal with the situation that we are sitting in the same boat as the patient (which will become the new "normal"). We are less "outside" the patient's problem situation than we used to be.—This was also an issue during the pandemic and is an issue in current crisis areas.—It makes sense to practice an attitude of connectedness and human compassion while maintaining therapeutic roles. If necessary, boundaries must be made explicit. Appropriate supervision or intervision for self-reflection and clarification of a possibly disturbing personal involvement is recommended.

Those specific features of PPT have a great potential to contribute to increase both climate justice and health protection: the general approach/image of the human being, the equal elaboration of similarities and differences in the transcultural approach, the balance model and, in particular, its fourth area, and the use of humor and stories.

The Approach—The Conception of Human Nature in PPT

Unlike all known crises and civilizational declines, the climate crisis is not a local problem but a global one. This is why globally networked, cross-national and cross-cultural cooperation is needed. A view of humanity that emphasizes the good core of people and their capacity for compassion and cooperation is very well suited to working on joint solutions across cultural differences. In a time of multiple crises, in which we cannot avoid fundamental transformations—change by design or change by disaster—this orientation helps us to remain constructive, resist resignation, and work together in a creative way. This applies on both a large and small scale, as well as in therapeutic work with people who are emotionally burdened by the climate crisis and the associated uncertainties. By a positive belief in mankind, including oneself, one can more easily stay confident even in times when we approach a climate catastrophe. There is always something one can do. Especially when one works together with others, that one does not see them as enemies or opponents but as co-creators of the solution. We are most effective when we focus on systemic levels such as economic, political, infrastructural levels, etc., and not only on personal consumption. This is more likely to succeed with a positive view of humanity and an approach of taking responsibility in the world.

The Transcultural Approach

In PPT we see all people as equal and essentially good, having things in common and differing from each other. There is a concrete set of tools that enable us to work out these similarities and differences. Every person has their own view of the world, the problems and potential solutions, each colored by their own socialization, culture, living, and experiences. With the help of basic and actual capacities we keep an eye on the fact that, despite all our differences, we have a common basis for being human that we can build on. To stay aware of that fact makes it easier for us to endure and regulate tensions. Our brain loves it when things are simple, predictable, familiar, and homogeneous. Diversity, on the other hand, forms the necessary creative and situationally adapted potential for multiple problem-solving. Being able to name and describe what is similar and what differs can reduce uncertainty and hostility. There will be no big master plan, but rather

many larger and smaller solutions, which in turn interact with each other, as so-called *heterotopias* (Welzer) [24]. Changes that work in one place or on a smaller scale can be scaled up or modified and adopted elsewhere. Appreciating the simultaneity of what connects and distinguishes can contribute to identifying as a "citizen of the world". This seems desirable, as research has shown that social identification with the world community plays a role in motivating cooperation that transcends parochial interests [25, 26, 27]. Social norms play a major role in controlling behavior as well. Various studies in Germany showed that much more people are prepared to engage in climate protection (43%) than they assume that others are (30%) [28]. This can easily reduce climate-friendly behavior. A vicious circle. Awareness and reflecting on my own culture accompanied by openness to other approaches, appreciating the differences, while feeling connected as a global citizen can promote collaboration and creativity.

The Balance Model

General Initial Considerations

The four areas of the balance model and the associated holistic view of life and suffering have already been presented at the beginning of this book (See Chap. 2). Of course, the climate crisis and its consequences have an impact in all areas and can be described and categorized using the model. However, with regard to the climate crisis, the fourth area is of particular importance. There we ask about values, convictions, ideas about the future, and how we deal with existential issues of life, death, and meaning. Particularly in the industrialized West of the global North, an unquestioned belief in progress and prosperity dominates, which suppresses and conceals the long-known massive destruction of nature and exploitation of people through our way of life and economy. Many people here believe that our prosperity can be attributed solely to our efficiency and other virtues. This goes hand in hand with not feeling particularly responsible or accountable for the suffering of others in other parts of the world—or even in our own country. "Every man forges his own destiny" is a fitting proverb. As the global nature of the climate crisis also has an impact on the "affluent North" and cannot be ignored in the long term, people are gradually starting to rethink, reflect, and question. From a psychological point of view, this is definitely to be welcomed, as the already unavoidable consequences of climate change lead to considerable suffering and the need to adapt. Society should try to re-invent itself by facing the facts instead of distracting or avoiding. The balance model is a useful tool to address and organize these issues. We know from our therapeutic work that understanding and categorizing can already provide relief and pave the way for ideas for coping and taking the first steps toward change.

The Balance Model in Therapeutic Work in and with the Climate Crisis

In view of the climate crisis and all the crises and uncertainties associated with it, the emergence of strong emotions, in particular fear, powerlessness, anger and sadness through to despair and resignation, is not surprising and is not pathological in itself. However, it can trigger, intensify, or modulate symptoms of mental disorders. In addition, it forms an important part of the background where mental illnesses take place, similar to the COVID-19 crisis or, in some places, war. This results in a certain tension between the therapeutic neutrality requirement on the one hand and the obligation to provide the most comprehensive health care possible for our patients on the other.

When taking an anamnesis, often not all relevant circumstances are mentioned, partly out of shame, partly because they are not conscious yet. As we know, that does not mean that they are not relevant. Since no one can completely escape this global crisis, it also plays some role in every therapy. This can be done indirectly, for example when professional concerns are linked to extreme weather events. Therefore below are questions about the climate crisis (and other crises), the associated feelings, and effects on personal lifestyle and symptoms in the anamnesis.

Questions can be, for example

- How do you see the future, both personally and socially?
- What do you expect to experience in the next few years?
- Are there things that scare you about the future?
- Which of the current crises are you concerned about (climate change, inflation, extinction of species, loss of social cohesion, war, general existential worries, etc.)? How does this make you feel? Does this affect your everyday life?
- What do you believe in? What are important values for you? What are you guided by in your life?
- Have you already lost important people or things? How did you cope with the loss?
- What gives meaning to your life?
- Are you involved in any form of voluntary or political activity to help shape a "better" future?...

Beyond the anamnesis and climate crisis-related issues that are broached by the patients themselves, it is a balancing act to what extent this health threat can be actively introduced by the therapist. It is a professional ethical duty of therapists to be aware of the health effects of the climate crisis and address this threat in a way that is appropriate to the patient's mental state and situation, always in combination with approaches for action. Experiencing individual and collective effectiveness in (social) engagement is a particularly effective coping strategy against feelings of powerlessness. Here too, the balance model can serve as a guide. On an individual level, for example, one can change eating habits or mobility behavior (body), choose sustainable service providers for electricity, banking or heating (performance), use equipment collectively, join cooperatives or associations (contact) and question my own personal convictions and plans (future, imagination, meaning). Of course, the same is also possible on a collective level, which is more promising in terms of effectiveness than a purely individualistic approach. The concept of the ecological handprint fits in with this—in contrast to the individualistic concept of the carbon footprint, which diverts attention away from the responsibility of large "players" [29]. The handprint is about having a social impact, for example by shifting norms toward more climate-friendly behavior or forming political will that leads to climate-friendly legislation. However, reducing your own CO2 emissions and resource consumption can be an encouraging start.

Humor and Stories in Dealing with the Climate Crisis

Narratives structure our world. We have known for a long time that our perspectives and expectations influence what we see and perceive. Even our imagination is framed by our beliefs. It is therefore of great importance how we narrate the present, past, and future. The work with stories, but also with metaphors, proverbs or humor, which is deeply rooted in the tradition of PPT (see Chap. 36), is based on the experience of their high effectiveness in therapeutic and developmental processes. Stories can take on a variety of functions. They can, as Nossrat Peseschkian liked to state: "Open the door to the imagination". And imagination and creativity are urgently needed to reduce and deal with the climate crisis. Stories help to adopt a different perspective, whereby indirect access via storytelling can often bypass defense mechanisms. Stories can encourage, act as a role model and have a repository effect.

We can distinguish between different qualities of stories. There are stories that can very generally help to make deadlocked thinking more flexible and open up new horizons, regardless of the specific topic. Many of the well-known short stories from Peseschkian's books are suitable for this [30]. In connection with the climate crisis, there is also literature and illustrated books that describe and visualize very concrete utopias of a climate-friendly world, like "Zukunftsbilder 2045" [31]. In both books and the internet you can also find a lot of inspiring stories from real projects [32, 33, 34]. As we are heading toward a completely changed world, stories and images

about a future in which people can live peacefully and adequately are extremely helpful, especially as there is an abundance of dystopian stories that can all too easily lead to resignation. Becoming aware of our culture's usual interpretation of concepts and narratives such as prosperity, progress, individualism, performance, justice, economy, freedom, etc. can also inspire new ways of thinking. Reflecting on values and what can really make people happy—as opposed to "buy yourself happy"—creates space for reorientation. In therapy, one can recommend the client to elaborate on the story of who they would have liked to be and ask them to what extent this corresponds to their own values and desires. One can also suggest writing their own eulogy or imagining similar future retrospectives in order to find a more future-friendly orientation in the presence. Furthermore, there are some stories published now that follow a different narrative structure to the usual hero's journey or other familiar storylines. They attempt to narratively depict the simultaneity and interweaving of levels and events, their interactions, their co-creative leaps, their self-organization and openness to the future, i.e., "Mushroom at the end of the world" [35]. Linear, causal thinking is gradually giving way to more complex, systemic thinking. Such stories may be suitable as recommended reading for in-between sessions, rather than for specific therapeutic work. Of course you can also do it as a self-education as a therapist. They also broaden the view, stimulate the imagination, and ultimately give hope that shifts in perspective and paradigms are possible and that the history of the future has not been completely written yet.

Finally, a few words about humor—see also Chap. 43. Humor is many things: a mature defense mechanism, an accepted way to release tension or express feelings, an expression of *joie de vivre*, something that connects people, the ability to distance oneself a little from one's own actions, the prelude to a creative solution. According to the situation we can make use of all of these functions. Often it can take weight off the situation for a moment without denying its seriousness. Here are two examples:

- Two people at a pub, having a beer. One: "These climate activists are all terrorists!" The other: "Yes, exactly, can't they just drink their beer like any normal person and sit on their sofas and wait for the apocalypse?" (unknown)
- "Life is an illusion. I have that in my employment contract" (W. Allen) [36]

Concluding Thoughts

Life is too short for a long face (N. Peseschkian) [37]

Climate crisis is here and causes a lot of (mental) suffering. We have an obligation to our own lives, the lives of other global citizens, and the lives of our descendants not to bury our heads in the sand. In addition, our (Western) society has a historical responsibility toward people and nature, particularly in the Global South, to recognize the suffering we caused and to strive to alleviate it. As psychotherapists and other health professionals in general we are experts in guiding people toward building up attachment and empathy (also with themselves). We show them how to cooperate, overcome obstacles, broaden their perspectives, question their beliefs and assumptions, and develop and change habits. We help them deal with feelings and conflicts. We accompany them in times of grief and assist them to let go. In the progressive West, traditions of saying goodbye and mourning have largely been forgotten. Cultural openness and learning from one another, appreciating diversity and differences inside and outside psychotherapy can help to build up skills that are useful in dealing with the upcoming catastrophe. In many ways we need a shift of paradigm to survive as humankind. There are already many good ideas and approaches. We strengthen both climate and health protection when we promote structural skills such as: Differentiating feelings, being able to talk about them and use them as a driver for action, communication and cooperation skills, creativity and dreaming, grieving, saying goodbye and welcoming new things. The use of stories is well suited to train the ability to mentalize, benefit empathetic understanding, and the connection

between people and between people and nature. The fact that we are part of a highly complex global system should not discourage us, but rather strengthen our hope that we can make an impact and initiate reorganization due to our interconnectedness. The time to act is now, both in prevention, transformation and curing, on individuals and society. Cooperating with other health professionals on a national and international level can be very beneficial both to ourselves, our patients, and society [38–40]. Giving up is no option. Looking back in the future, who do we want to have been? PPT offers various tools and a general approach that can help us to fight the crises and develop toward a peaceful, ecological, and more just world.

> Hope is unlike optimism, not the belief in a good outcome but the conviction that everything makes sense whatever it will look like. (V. Havel) [41]

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45 Dealing with the COVID-19 Pandemic with Positive Psychotherapy

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Since the end of 2019, Coronavirus Disease 2019 (COVID-19) has spread rapidly across the globe. The COVID-19 pandemic has seriously disrupted people's normal work, study, and daily life, and has had considerable negative impacts on both body and mind. As the COVID-19 pandemic progressed, the public faced immense physical and psychological threats. The global health crisis caused by COVID-19 has proven to be more destructive and enduring than most people anticipated. Multiple factors, including drastic changes in social life, lack of social interaction, disruption of social support systems, fear of infection, and perceived discrimination after quarantine, have affected the physical and mental health of the public [29]. Numerous studies have reported on the impact of COVID-19 on physical and mental health. Physical health influences include weight gain, emotional eating, decreased sleep quality, and symptoms of neurological diseases. Mental health impacts include anxiety, depression, post-traumatic stress disorder (PTSD), increased suicidal ideation, and COVID-19-related phobias [32]. The impact of COVID-19 extends beyond the general population, significantly affecting the mental health of specific vulnerable groups, with only a portion of individuals possessing good psychological resilience able to "bounce back" quickly from stressful environments [16].

COVID-19, an infectious disease, was unfamiliar to most people at its inception. In an atmosphere filled with the unknown and fear, "hope" becomes more precious than gold. "Hope" is one of the three principles of Positive Psychotherapy. In Positive Psychotherapy (PPT after Peseschkian), "positive" refers to what is already present, real, and whole. Anything that exists in your life is a resource [20]. During the COVID-19 pandemic, people immersed themselves in fear of the unknown, hoped for the end of the pandemic, wished for their own recovery, and wanted the stability of social order. These are all ways in which individuals "positively" cope with the changes using their own methods. However, if one's coping mechanism is too singular and rigid over time, its effectiveness will diminish, and it may even create new conflicts. Positive Psychotherapy, as an integrative and cross-cultural therapy, advocates for incorporating various perspectives to find new methods that can provide personal assistance. It also promotes collaboration among doctors, therapists, educators, and social workers in responding to COVID-19. It encourages the cooperation of different therapeutic methods in psychotherapy.

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_45

Influence of COVID-19 on Physical Health

Symptoms of COVID-19

COVID-19, as a global infectious disease, has widespread and profound impacts on people's physical health. The symptoms of COVID-19 can be categorized as mild or severe. Mild symptoms include fever, dry cough, and fatigue. In more severe cases, patients may experience difficulty breathing, acute respiratory distress syndrome (ARDS), acute cardiac injury, and multiple organ failure [7].

Pathology of COVID-19

The pathological manifestations of COVID-19 in the human body are also very complex. It can affect the cognitive system, leading to poor mental state, difficulty concentrating. During the acute and chronic phases, cognitive decline such as executive dysfunction, memory impairment, and slowed processing speed may occur [47]. The virus is mainly detected in alveolar cells and epithelial progenitor cells, primarily affecting the respiratory system, causing significant alveolar damage and edema, which may lead to respiratory failure [7]. Neurological complications are also common in COVID-19, including headaches, loss of smell, confusion, and disabling strokes. The virus may enter the brain through the olfactory system, leading to loss of smell, and it may affect other brain regions through retrograde transport and trans-synaptic spread. COVID-19 can also damage the kidneys, liver, gastrointestinal tract, and other endocrine organs, causing systemic metabolic changes that can nonspecifically affect the nervous system, such as causing confusion [10]. The impact of COVID-19 on the nervous system may involve direct neural invasion and indirect systemic factors. COVID-19 can damage bodily systems during both the onset and recovery phase. For example, during the onset phase, viral infection may cause heart damage, myocardial injury, and arrhythmia, affecting the cardiovascular system [10]. Another study mentioned that some patients experienced ischemic stroke and acute pulmonary embolism due to deep vein thrombosis in the lower limbs after discharge [27]. Research found that older patients had higher chest radiographic scores after discharge, indicating that elderly patients may be more susceptible to persistent lung damage [45]. Based on existing clinical and basic scientific investigations, it can be recognized that the pathological impacts of COVID-19 on the human body systems are multifaceted.

Sequelae of COVID-19

COVID-19 infection can also lead to many sequelae which cannot be ignored. Some recovered patients may experience issues such as long-term fatigue, breathing difficulties, and psychological disorders [8], negatively impacting their quality of life and health status. A study focusing on the long-term health consequences of COVID-19 six months after discharge in residents of Wuhan found that fatigue or muscle weakness was one of the most common symptoms, affecting 52% of recovered patients. Sleep problems were reported by 437 patients, accounting for about 26%. Additionally, lung function impairment varied according to the severity of the disease, with impaired lung diffusion capacity ranging from 22% (mild disease) to 56% (severe disease) [27]. A study in Mexico reported 27 different sequelae symptoms, with the most frequent symptoms such as difficulty concentrating (81%), breathing difficulties (75%), joint pain (71%), fatigue (68%), and hair loss (60%) [21]. These data indicate that recovered COVID-19 patients may suffer from a range of long-term physical and psychological sequelae, and the incidence of these sequelae is related to the severity of the disease.

Relationship Between COVID-19 and Mental Health

Impact of COVID-19 on Social Psychology

The COVID-19 pandemic has not only affected the global economy and society and severely

damaged people's lives and physical health, but it has also had a significant impact on people's psychological well-being. A survey conducted by the Social Psychology Research Center of the Institute of Sociology of the Chinese Academy of Social Sciences during the first major outbreak of COVID-19 revealed that the public generally experienced anxiety and that a high proportion of the population exhibited emotions such as fear, anger, sadness, and panic. Social media became the main channel for disseminating information about the pandemic [34]. The complex information prevalent online exacerbated anxiety and other negative psychological states among the public [35]. As a collective and intersection of individual mindsets, social mentality further influenced the level of public trust [31]. However, studies have found that people who have recovered from COVID-19 tend to show increased trust in strangers, possibly due to receiving more selfless help and social support [3].

Effects of COVID-19 on Individual Psychology

The outbreak of the pandemic, fear of the unknown virus, worry about quarantine, restricted activity spaces, lack of social activities, risk of unemployment and economic pressure, and concerns about infection have all become significant factors causing immense psychological distress to the public [9, 30]. Individual psychological states generally undergo these four phases: panic phase, defensive phase, adaptation phase, and recovery phase. However, each stage has distinct psychological stress response characteristics [24]. Notable psychological issues include an increase in psychosomatic disorders, low mood, persistent fatigue [41], significantly elevated levels of depression and anxiety, disrupted sleep quality, and increased negative behaviors [33]. Those impacted by the pandemic or who have lost loved ones to the disease are also more likely to experience post-traumatic stress disorder [25]. Therefore, all sectors of society have given great importance to mental health issues.

Sleep Quality

The COVID-19 pandemic has had a significant negative impact on people's sleep. A review of both domestic and international literature reveals that the pandemic has led to the emergence and worsening of sleep problems across various age groups and professions worldwide [18, 28]. Among these groups, those most affected are COVID-19 patients, which may be related to the side effects of medications and physiological changes associated with the illness [28]. Within professional groups, healthcare workers are the most impacted, with multiple studies indicating that their sleep quality is poorer and insomnia risk is higher compared to other professions, especially among women [6]. Additionally, researchers have studied sleep conditions across different age groups, finding that sleep quality has declined across all ages. This decline may be attributed to the lockdown measures adopted during the pandemic, which likely disrupted circadian rhythms due to reduced daily activities, changes in eating habits, increased negative emotions, and stress [28]. Among these groups, the elderly are particularly affected. For older adults, the factors contributing to sleep disorders are many. The most important is the poor information access in rural areas, leading to inadequate knowledge about COVID-19 and resulting in panic and other negative emotions, which in turn cause insomnia and other sleep disorders [39].

Anxiety and Depression

Under the individual-level impacts of the COVID-19 pandemic, an increase in anxiety and depression is another very notable effect [6]. The World Health Organization's "2022 World Mental Health Report" cites data from the "2020 Global Burden of Disease Study," indicating that during the first year of the pandemic, the global incidence of anxiety and depressive disorders increased significantly by 27% and 25%, respectively, with approximately 76 million and 53 million cases [38]. As the COVID-19 pandemic unfolded, individuals in different age and occupational groups exhibited varying degrees of anxiety and depression. The emotional and affective states of individuals are influenced by a com-

bination of environmental characteristics of specific periods and individual factors.

The impact of the pandemic on anxiety and depression varies across different age groups. Research has found that the suddenness and severity of the pandemic, when interacting with the still-developing adolescents, lead to increased issues of depression and anxiety among them [37]. Similarly, under the stress of localized outbreaks and lockdown measures, the sudden changes in learning and living conditions and the interruption of regular social activities have negatively impacted the mental health of university students, with levels of anxiety and depression significantly higher during the pandemic [5]. An article published in October 2023 in the "National Depression Blue Book" mentions that 30% of depression patients are under 18 years old, and 50% of depression patients are students. This trend of increasing depression among younger individuals highlights the urgent need to address the mental health of the younger generation. Additionally, most literature suggests that elderly individuals, as a highly susceptible group, experience a slight increase in anxiety and depression levels due to difficulties in interpersonal relationships and family responsibilities (such as loneliness, fear, and uncertainty about the pandemic). However, compared to younger people, their overall mental health status is relatively better [4, 19].

Regarding the impact of the pandemic on anxiety and depression across different occupational groups, the outbreak has affected various professions to different extents. A review of the literature shows that many researchers have focused on frontline healthcare workers and educators, who are key in future talent development. Healthcare and health workers, as the main force in pandemic control, have garnered significant attention from researchers. They were challenged by physical and psychological stress, such as high risk of infection, heavy rescue tasks, and challenges in securing protective equipment, leading to feelings of depression and anxiety [17, 46]. Given the unique role of teachers, monitoring their mental health is crucial amid significant changes in their professional practice. A systematic review assessing the prevalence of anxiety, depression, and stress among school and university teachers during the COVID-19 pandemic noted that university teachers reported lower levels of anxiety and depression compared to primary and secondary school teachers. This may be because university teachers have more experience with remote education, and university students are more easily adapted to remote learning compared to children and adolescents [22]. In summary, the pandemic has led to varying degrees of increased anxiety and depression across different age groups and professions. Given the potential for long-term harm, it is necessary to implement psychological intervention measures.

PTSD

Researchers have also focused on post-traumatic stress disorder (PTSD) that emerged after experiencing the COVID-19 pandemic. Overall, the pandemic has led to PTSD in various age and occupational groups, and exacerbated symptoms in those with a history of PTSD or comorbid conditions [23]. Firstly, for children, the government's lockdown policies to control the viral spread might result in separation from their parents, which can lead to PTSD [13]. For adolescents, the shift from in-person to online learning due to the pandemic may cause adaptation issues, leading to various negative emotions. Additionally, deprivation of outdoor activities, separation from peers, and an increase in domestic violence due to lockdowns are factors contributing to PTSD in this age group [15]. For elderly individuals, especially those with comorbidities, the negative impact of lockdowns is particularly severe, potentially triggering a range of fears and ultimately resulting in PTSD after the pandemic [40]. Regarding different occupational groups, the causes of PTSD also vary. For university students, some scholars believe that the breakup of family relationships and significant economic losses caused by the pandemic may lead to PTSD [36]. For healthcare workers, despite their extensive experience and medical skills, the immense work pressure and high patient mortality rate during the pandemic have led to a high

rate of PTSD among them [12]. For COVID-19 patients, despite prolonged treatment, the persistent negative impacts of the disease mean that while the PTSD detection rate has decreased, it still exists. Hence, some scholars suggest that psychological interventions are necessary during the long-term recovery for COVID-19 patients [26]. Additionally, other studies have found that the pandemic can exacerbate PTSD symptoms in individuals already suffering from PTSD, making psychological intervention and treatment particularly important [23].

Other Psychological Symptoms

Aside from the aforementioned mental diseases, there are many other psychological issues that individuals have experienced as a result of the pandemic, which have received considerable attention from the academic community. Iob collected data during the first month of the pandemic in the UK and found that women, Black and minority ethnic groups, as well as individuals facing economic issues, unemployment, disability, chronic illness, mental health conditions, and COVID-19 diagnoses, had a higher frequency of suicidal and self-harming thoughts [11]. These aggressive thoughts may also shift towards external targets due to boredom during isolation, depending on the individual's cognitive flexibility [14]. Additionally, frontline healthcare workers, burdened with dramatically increasing workload, are particularly susceptible to burnout, especially if they already have low self-esteem, which may lead to negative coping strategies [17]. Other studies have confirmed that adopting negative coping strategies in the context of the pandemic can hinder individuals' recovery from mental health issues [2]. The psychological conditions of children and adolescents during home-based online learning have also been widely studied. Many researchers have found that prolonged engagement in this mode can lead to adverse lifestyle habits, such as internet addiction, disrupted routines, and decreased physical activity. Combined with potential family discord, this can further exacerbate psychological problems in children and adolescents [42, 44].

Positive Psychotherapy Framework to Cope with COVID-19 Pandemic

An Integrative Concept of Positive Psychotherapy—Five-Finger Alliance

Through a review of domestic and international studies related to the COVID-19 pandemic, it is evident that COVID-19, as a major public health crisis, has impacted the public both physically and psychologically. It is noteworthy that, in addition to employing pathology-oriented interventions focused on problem rectification, there should be a significant emphasis on positive psychology and positive psychotherapy interventions aimed at enhancing psychological resilience. This can be achieved through methods such as maintaining social interactions and accepting negative emotions to foster positive emotions and enhance positive qualities, thereby improving the ability to cope with sudden public health emergencies. Therefore, it is crucial to encourage interdisciplinary collaboration among epidemiologists, psychiatrists, psychologists, sociologists, and neurobiologists. Additionally, preventive and intervention measures should be incorporated into future national emergency management laws and regulations to build a comprehensive public health response mechanism focused on physical and mental well-being.

From the perspective of positive psychotherapy, COVID-19 impacts all four areas of an individual's life. The systemic effects of COVID-19 on a patient's physical, psychological, and social functions also mean that it poses challenging difficulties for a single professional, whether a psychiatrist or a psychotherapist, when attempting to assist the patient. To address this challenge, we have proposed the "Five-Finger Alliance." It is a metaphor. Just as the strength and function of a fist are far greater than that of a single finger, we compare the patient and their supporting system to the thumb, which is the greatest force for successful psychotherapy. Psychiatrists and psychosomatic doctors are likened to the index finger, which can provide diagnosis and medication, much like how the index finger is always used to point the way. Psychotherapists are compared to the middle finger, the longest finger on our hand,

symbolizing our resources; psychotherapists accompany the patient in seeking and using various resources in their life. Educators are likened to the ring finger, which, when wearing a wedding ring, evokes a sense of responsibility and commitment; educators can provide patients with basic psychological knowledge, developmental stage knowledge, and academic guidance. Social workers are compared to the little finger, as social workers can work as flexibly as the little finger, providing psychological support and assistance to patients and their families in their communities. Positive psychotherapy, as an integrative therapy and meta-therapy, offers a common working language for doctors, psychotherapists, educators, and social workers. Different roles can collaborate and consult from the perspective of positive psychotherapy to jointly help the patient. When the "five fingers" form a hand, its strength far exceeds the mere sum of the five fingers, creating infinite possibilities. This is also the limitless potential brought about by the cross-cultural cooperation advocated by positive psychotherapy, which is helping us solve the difficulties and dilemmas encountered in psychological work. The "Five-Finger Model" emerged from the difficulties brought by COVID-19, as Professor Peseschkian said, difficulties have a positive side, as they foster our ability to cooperate and make us accept that we are not omnipotent [1].

Case Conceptualization by Positive Psychotherapy

Language is crucial for communication. Providing a common language for psychiatrists, therapists, educators, and social workers when jointly assisting a patient is essential. Positive psychotherapy, as a structured psychodynamic therapy, features a structured and logical mode of expression. At the same time, its humanistic and cross-cultural philosophical perspective allows each role within the "Five-Finger Alliance" to maintain their professional uniqueness while being understood by collaborators. In the case of conceptualization of the "Five-Finger Alliance," we also drew from the Operationalized Psychodynamic Diagnostics (OPD) approach, utilizing the four dimensions of symptoms, relationships, conflicts, and personality.

We demonstrate the conceptualization through the case of a female patient who developed psychological issues after COVID-19 infection.

The 21-year-old patient, who should have been attending college in another city, took a leave of absence due to concerns about her health. She is introverted and meticulous, and also performs well academically. Before the pandemic, she was a freshman in college, just beginning to adapt to university life. With the outbreak of COVID-19, her university switched to online classes, and she returned home to study remotely. Later, when the university resumed in-person classes, her peers returned to campus, but she was reluctant to go back due to fear of infection. After contracting COVID-19, she began experiencing symptoms such as headaches, insomnia, forgetfulness, body aches, and fatigue. Over time, these symptoms worsened, and she refused to return to school. Her parents' persuasion failed, and she was unwilling to communicate with them. Her college counselor reached out to inquire if she would take the final exams, but she declined to answer the calls. Although her parents set return dates for her to go back to school, her symptoms would worsen as the dates approached, and her biological rhythms deteriorated to the point where she could not eat, sleep, or bathe on schedule. Eventually, at the counselor's suggestion, her parents took her to the hospital for treatment.

"Symptoms" Axis

The patient felt extremely fearful about returning to school, which caused her to have nightmares and poor sleep quality. Due to inadequate rest at night, she felt lethargic and uninterested in activities during the day. The patient had contracted COVID-19 three times, each time experiencing severe physical distress (fear of death). She was highly averse to being infected again and disliked crowded places, thus perceiving the school (especially the dormitory) as a very dangerous place. The mere thought of school made her tense, unable to concentrate, and sometimes she felt a vague discomfort in her chest.

In PPT, we emphasize the function of symptoms, their symbolic meaning, and positive interpretation. The patient can be likened to a "fawn drinking water by the river," needing to stay alert for any "predators" that might attack while she drinks. Anxiety acts as a "sentry," reminding her how to stay out of danger and survive.

After the COVID-19 pandemic, the patient developed anxiety due to fear of death and uncontrollable circumstances. The physical discomfort she experienced after infection further "fed" her anxiety, producing a snowball effect.

"Relationship" Axis

The patient had poor early attachment, primarily living with her grandparents. The elderly grandparents lacked the energy to care for her adequately, instilling in her the belief that "it is safer for girls to stay at home." As a result, the patient feels anxious about unfamiliar environments.

Her secondary capacities have developed well, but her primary abilities are underdeveloped, leading to a lack of rich emotional communication and experiences.

Through the model dimension, it is observed that the patient's mother is very anxious, often expressing worries about daily life and academic matters, which taught the patient to "be anxious" from a young age. After the outbreak of COVID-19, the patient was extremely concerned about her parents contracting the virus while treating patients, fearing for their safety. Even after the pandemic ended, she continued to feel anxious in crowded places, fearing infection. The patient's university and major were chosen by her parents, who hoped she would become a doctor. However, after COVID-19, she realized the dangers of the medical profession and began to resist pursuing a medical career, frequently taking leave from classes. Her teachers perceived her as lazy and uninterested in studies, and her classmates thought she was strange, leading to poor social relationships and frequent meetings with teachers, making her increasingly reluctant to stay at school.

From a social perspective, the online class format limited her opportunities to interact with teachers and classmates, resulting in little social support.

"Conflict" Axis

In the patient's childhood, due to her introverted temperament and early attachment experiences living with her grandparents, she tended to develop the belief that "home is safe, and everything outside is dangerous." Loyalty to her family of origin provided her with a strong sense of security and hope in life. In positive psychotherapy, we use "hope-loyalty" to describe the patient's basic conflict. This basic conflict played a key role in establishing her sense of security and attachment early on. Her choice to attend medical school was also an act of loyalty to her parents' aspirations in the area of achievement.

However, the COVID-19 pandemic, like a trigger, renders the patient's basic conflict (previously effective beliefs) ineffective. In the patient's mind, her parents were seen as omnipotent saviors, but during the pandemic, they appeared helpless and exhausted in reality. Her parents, as healthcare workers, also exhibited fear of death, which activated the patient's deep-seated fear of death. The pandemic brought a strong sense of uncontrollability, making the future unpredictable. This fear of death and the sense of uncontrollable change constitute the patient's reality conflict.

Thus, we see a set of opposing drives in the patient. On one hand, she no longer wishes to be loyal to her family's career choice of becoming a healthcare worker, facing the fear of death she cannot accept. On the other hand, if she deviates from the career path her family planned for her, she must make her own life choices, all of which are filled with uncertainty and uncontrollability. This "Neither…nor…" dilemma, described as an inner conflict in positive psychotherapy, is the patient's inability to choose either path.

The patient's inner conflict is ultimately represented through her illness "social phobia" which helps her retreat into her inner world, where she no longer has to make decisions about her social career and is shielded from the impact of her inner conflict.

In OPD, this conflict is described as "dependence VS individualization" conflict.

"Personality" Axis

The client exhibits an anxiously avoiding neurosis disposition. She lacks a sense of self-security, needs the companionship of others, and avoids unfamiliar situations and relationships. Her unconscious expresses the expectations like: "Always be there for me, don't leave me alone; tell me that everything will be fine; keep reassuring me that nothing can happen to me." Her unconscious basic conflict is: "I want total security, but nothing and no one is safe for me, so I have to be constantly on guard that I am not alone and unprotected." When doctors or therapists work with this patient, their countertransference shows, at first sympathy and willing to take the responsibility of auxiliary-I function, but at some point, they become helpless because no one can guarantee total security.

The Five-Finger Alliance Working Method of Positive Psychotherapy

Through positive psychotherapy case conceptualization, psychiatrists, therapists, educators, and social workers can collaborate on a single case. For many years, we have been attempting to facilitate in-depth collaboration among psychiatrists, therapists, educators, and social workers who are familiar with positive psychotherapy.

We divide the work into three stages. The *first stage is the preparation and testing* phase of treatment, during which educators and social workers often play a "sentinel" role. This is achieved by providing educators and social workers with basic theory of psychology, psychopathology, psychodynamics of mental disorders, and positive psychotherapy. If the patient is a student, educators often serve as the "whistle blowers" who first detect psychological problems in the patient. If the patient is an adult, social workers (some companies have in-house counselors) can assume the "sentinel" role. In this case, the patient was identified by a university counselor who had been trained in positive psychotherapy. The counselor conducted a 30-minute interview using positive psychotherapy techniques and, after assessing the patient's body, achievements, contact, and future, recommended that the patient visit a hospital. With the patient's consent, the counselor can provide the psychiatrist with a structured overview of the patient's situation in the four areas above via phone, email, or letter to construct the psychiatrist's assessment.

In our work model, patients at the hospital meet with both psychiatrists and psychotherapists (or psychological counselors). During the initial diagnosis and treatment of mental and psychosomatic disorders, in addition to the regular medical work, psychotherapists provide optional psychological education and counseling services. The purpose of this phase is to educate patients about their situations, the relationship between themselves and their illnesses, and to provide a basic understanding of psychological counseling. Psychotherapists use tools from positive psychotherapy to conduct systematic and semi-structured psychological education. This involves a model of 3 fixed-topic sessions plus 5 sessions based on the patient's needs.

The first psychological health education session focuses on how patients can cope with stress and conflict. Using 12 items from the WIPPF scale related to conflict reactions, patients can visualize and understand the relationship between their mental health conditions, their lives, and their stress. The second session's theme is the energy distribution chart, which allows patients to proactively explore their lives and identify which factors are contributing to their problems. The third session emphasizes the positive connotation of symptoms, helping patients realize that they are not entirely passive in their illness, thus fostering a sense of agency in their recovery. Sessions 4 to 8 are tailored to the needs of the patients. Some patients may need to be listened attentively, others may require an analysis of the causes of their illness or conflicts, and some may need practical solutions to their problems. For patients who need to be listened to, the psychotherapist uses primary capabilities to provide the necessary emotional support and employs the three-stage interaction model to establish a relationship. For patients who need to analyze the causes of their illness or conflicts, the psychotherapist uses the Differentiation-Analytic Inventory (DAI)—see Chap. 2—inventory to discuss their actual conflict and key conflicts. For

those seeking problem-solving methods, the psychotherapist applies the five stages of session in positive psychotherapy to find solutions and may also work on training the patients' actual capacities and addressing key conflicts.

The *second phase is the formal treatment stage*. During this phase, while patients continue stable medication treatment, they receive assistance through positive psychotherapy. With the patient's consent, educators and social workers may also be involved in this phase. These four roles will also participate in regular peer supervision and expert supervision within the framework of positive psychotherapy.

In this case, the psychiatrist and psychotherapist will first focus on the psychodynamic causes of the patient's anxiety symptoms, providing a positive interpretation of his anxiety, and working together to identify what could replace his anxiety in order to release the patient's fear and sense of losing control. While the psychiatrist stabilizes the patient's symptoms with medication, the psychotherapist can concentrate on the patient's conflicts, provided that the patient has clear self-awareness and stable mentalization. The psychotherapist aims to help the patient address their actual conflicts, basic conflicts, and inner conflicts. The focus of the psychotherapist is on the development of the patient's actual capabilities, discussing which aspects of these capabilities have facilitated the patient's adaptation to past life experiences, and identifying any insufficient or excessive developments. The psychotherapist will explore how these capabilities that once benefited the patient in past experiences are now limiting their self-care, understanding of relationships, energy distribution towards achievements, or flexibility in future planning.

Meanwhile, the student's counselor invites the patient to participate in a university group session focused on "Enhancing Social Skills," aiming to help the patient gain primary capabilities like "trust" and "contact" with the support of social dynamics. The counselor also encourages the patient to join university film and drama clubs, hoping the patient can learn new ways to release fear and feelings of losing control in the "future area."

The social worker can enter the patient's home to give psychoeducation to the parents and help them understand why their child is ill. Additionally, the social worker assists the parents in gradually releasing their repressed fears of death related to the COVID-19 pandemic. By using DAI, the social worker helps the parents see their child's lack of trust and contact capacities and reflect on their own primary and secondary capabilities. The social worker, serving as a role model for primary capabilities, conveys the emotional development needed to the family. The social worker accompanies the parents in understanding the meaning behind the child's symptoms, genuinely listening to and understanding the child, and guiding the parents to bridge the gap with the child. This involves adopting an open and accepting attitude and engaging in honest, in-depth communication with the child. The social worker helps the parents practice patience and acceptance in their interactions with the child, listening attentively without criticizing or judging, and simply reflecting on their current feelings, expressing understanding and acceptance towards the child.

The *third phrase is the prognosis stage*. Once the patient's symptoms have largely resolved, the physician and psychotherapist need to focus on building the patient's psychological resilience to prevent future relapses under stress. Reflection is crucial during this stage. The physician can assign tasks for the patient to review their illness journey and reflect on how they helped themselves recover from the illness, enabling the patient to become their own doctor and expert. Additionally, the physician can discuss with the patient what steps they can take in the future to better manage themselves should they encounter illness again.

During this stage, the psychotherapist can work with the patient to review how they have resolved conflicts, improved relationships, and developed their structure level during past treatment experiences. The psychotherapist can also discuss with the patient how to use the five steps of positive psychotherapy for self-help in the future. Additionally, they can explore which types of events might represent the patient's

future actual conflicts and discuss strategies for coping with these conflicts in their future life.

Educators and social workers play crucial roles during this stage, as they assist the patient in re-engaging with society. Educators will discuss with the patient how to return to school life, including what has happened in the school and class during the patient's absence and how the patient can reintegrate into the class. They can also help reduce the stigma associated with mental illness and work to de-stigmatize mental health issues within the school environment. Educators might organize psychoeducation groups about mental disorders, transforming individual illness and crises into collective growth opportunities, and enhancing the class members' ability to cope with emotions such as anxiety.

Social workers will undertake similar tasks within the community. In addition, it is vital to provide ongoing support and assistance to the affected family. In cases like this, where the patient has an anxious personality, there may still be challenges in the future from sudden threatening events. Social workers, through their extensive support for the family, have accumulated valuable experience in helping them manage these challenges.

The concept of diversified positive mental health movement has become a trend in the development of mental health work. Positive psychotherapy serves as a "connecting bridge," particularly in cross-cultural contexts. In positive psychotherapy, different cultures offer various resources, and cross-cultural conflicts are seen as both a major source of psychological problems and a resource for solving them. This perspective provides theoretical and methodological support for cross-industry collaboration. The COVID-19 pandemic has posed significant challenges to humanity, and it is hoped that this challenge will ultimately promote cross-cultural cooperation and exchange.

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46 Understanding LGBTQ+ People's Needs in Therapy from a Positive Psychotherapy Perspective

Raluca Ursica

Background

In the complex tapestry of human experience, psychotherapy has evolved to recognise and celebrate the diversity of people seeking help and healing. As we dive deeper into the complex landscape of psychotherapy with LGBTQ+ people, it becomes critical that we approach our work with cultural sensitivity, humility, and a dedication to inclusivity.

The world is changing and moving towards greater acceptance and understanding of different identities. The importance of adapting psychotherapeutic interventions to the needs of LGBTQ+ communities has become more relevant. In this chapter, we explore the complexities of delivering affirmative and competent care by recognising the intersections of gender identity, gender expression and sexual orientation.

Our perspective of mental health and well-being must reflect the diversity of the people we serve. It is a path of self-reflection and professional development, as we attempt to become allies and advocates for individuals who have historically been marginalised and discriminated against.

As we traverse the complexities of psychotherapy with LGBTQ+ people, let us begin this chapter with an open mind.

Labels

In any society, ideas about sex, gender, and sexuality are highly culturally and historically determined, and the language that expresses these ideas is profoundly influenced by that.

LGBTQ+ stands for lesbian, gay, bisexual, transgender, queer/questioning, and related communities, plus everyone else who is not heterosexual or cisgender. Other letters and symbols are often added to the acronym and can include I for intersex; A for asexual, aromantic or allies; P for pansexual.

Lumping everyone who is not cisgender or heterosexual into an acronym may be a one-size-fits-all way of looking at identity; however, this approach is still a step too far for some people. While using the entire acronym demonstrates goodwill, it is clumsy and marks the people whom it attempts to represent as "not real" or "other", which may not be the direction in which we want to move. The term "queer"

To be fully seen by somebody, then, and be loved anyhow—this is a human offering that can border on miraculous.
Elizabeth Gilbert [1].

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_46

has been reappropriated by some as their umbrella term.

The acronym LGBTQ+ has been adapted into various forms since it was first used in the early 1990s. Activists, scholars, and community leaders have expanded the acronyms to reflect the complex makeup of the community. As more people identify with different orientations or sexual characteristics, the acronym expands to include them. Adding and reordering the letters in the acronym reflects progress and inclusivity. When the "Q" was added for "Questioning" or "Queer," it reflected the growing number of younger people who did not feel a need to be labelled.

The usefulness of using labels, identity concepts, and taxonomies in psychotherapy with LGBTQ+ clients serves as an interesting debate. The necessity for labelling LGBTQ+ individuals is a reaction to living in a largely heterosexual society, as well as labels that have been enforced on queer individuals to trivialise them. Identity concepts differ when labels are assigned to people by others compared with when individuals choose their own labels [2].

Using labels can help validate and affirm the client's identity, fostering a sense of self-acceptance and authenticity; in other words, labels help make diverse identities visible and acknowledged, leading to empowerment and a reduction in feelings of invisibility and marginalisation. As mentioned above, words are powerful and shape communities' perceptions of gender identity, gender expression and sexual orientation. Shared language facilitates communication and understanding within and outside LGBTQ+ communities. It helps people identify and connect with others and find networks that share similar experiences and identities. Labels can aid in advocacy efforts, helping highlight specific problems faced by different groups within LGBTQ+ communities.

From a therapeutic perspective, helping clients "own their label" can be an empowering act of self-definition and resistance to social norms. Knowing a specific label can help therapists provide a framework for discussing issues pertinent to their client's sexual orientation, gender identity, and the intersection of these aspects with other areas of their lives.

There are also controversial arguments for using labels such as the oversimplification of a person's experience and complex fluid identity. In a way, boxing them into a category that does not fully represent who they are. Clients might feel pressured to choose a label even if they are still exploring their identity, which can be stressful and restrictive. A label can easily lead to assumptions, resulting in a misinterpretation of the client's needs and experiences. There is also a risk of pathologising identities when labels are used in clinical settings, treating nonheteronormative identities as deviations and reinforcing internalised homophobia in therapy (see term transsexual). Of course, clients have the authority to reject or accept labels on the basis of what they feel and think rather than being assigned a label by their therapist. Sometimes they prefer to be seen as unique individuals, without the confines of a label.

Balancing the use of labels in therapy means allowing the clients to define their own identities and decide which label, if any, they feel comfortable with. This client-centred approach means that the therapist respects the fluidity of identities and is open to changes in how the client describes themselves over time. Labels should be used as tools of understanding, not as defining categories, to help the client feel empowered and seen.

Sex Assigned at Birth vs Gender Identity vs Sexual Orientation vs Gender Expression

To better understand the complexities of the LGBTQ+ client, we need to clarify what sex assigned at birth, gender identity, sexual orientation, and gender expression are.

As positive psychotherapists, the easiest way to imagine these concepts is by using the balance model framework [3].

Physical

Sex assigned at birth (SAS) can be understood as a biological and physiologically based characteristic that is dichotomised into male (XY) and female (XX) bodies. Although there is variability in physiological, somatic, and chromosomal development, the treatment of these bodies is commonly depicted in binary, oppositional, and discrete system categories.

Mental

Gender identity represents a deeply held, internal sense of the bodily self. It encompasses how individuals feel and identify personally in terms of their gender: female, male, some of each, or neither. Gender identity is often ingrained and becomes increasingly salient throughout childhood. By age 3, most children start to label themselves and others as either girls or boys. Interestingly, in a study conducted by Zaliznyak in 2021, more than 75% of the participating transgender adults could trace a diagnosis of gender dysphoria or cross-gender wishes back to early childhood. However, for some, gender identity does not fully fuse until adolescence or early adulthood, but once it does, it is part of the individual's core identity. As an individual progresses through life, their gender identity can develop and even change over time [4].

In essence, gender identity represents a set of diverse meanings that grow, adapt, and change alongside an individual while being deeply connected to social encounters. Consequently, adolescents and young adults who face developmental disruptions in gender identity (e.g., transphobia) are forced to confront many related symptoms. Several sexual minorities who do not fit into the expected social categories (i.e., engage in same-sex sexual activity, self-identify as transgender) have early adolescence fraught with conflict and gender dysphoric concerns. Within adolescence, a number of these individuals may be concerned with how, what, and when to share their gender history with their selected peers and romantic partners. As these clients enter emerging and young adulthood, there is a significant increase in presenting for gender treatment and for couples therapy where one patient has a gender history.

It is crucial to acknowledge the broad scope of how gender is expressed in the world. Typically, there are three main manifestations of gender identity: cisgender, transgender, and nonbinary. However, these are descriptions of people's souls rather than clinical definitions of their existence, which can lead to misunderstandings for people who organically express multiple descriptors. A greater degree of trauma is correlated with higher levels of gender-related stress. It is important to think about trauma and how parents express or do not express their gender to their children. If they exploit their masculine or feminine traits or only bring one part of their expression forward (manly man, obedient wife, etc.), their children could be determined to repeat this process [5].

Let us explore in detail the three general gender identities mentioned earlier: cisgender, transgender, and nonbinary.

Cisgender (often abbreviated as "cis") is a term used to describe individuals whose gender identity matches the sex they were assigned at birth. For example, if a person is assigned female at birth and identifies as a woman, they are considered cisgender. The term is used to differentiate between those whose gender identity aligns with their birth-assigned sex and those whose gender identity does not (e.g., transgender, nonbinary). Cisgender is derived from the Latin prefix "cis-", meaning "on this side of," whereas "trans-" means "across" or "on the other side."

Transgender, or trans, is a broad term used for individuals whose gender identity does not align with the sex they were assigned at birth.

Most transgender individuals are not nonbinary. Although some transgender people identify as nonbinary, the majority have a gender identity that is either male or female and should be regarded as any other man or woman.

Some transgender individuals may choose to undergo gender-affirming medical procedures to align their bodies with their gender identities. While not all transgender people require medical interventions to live fulfilling lives, for many, these procedures are essential and life-saving.

While the term "transgender" and its current definition emerged in the late twentieth century, people who fall under this category have been present in many cultures throughout recorded history.

Here are some examples: Galli in ancient Greece and Rome were eunuch priests dedicated to the Phrygian goddess Cybele (known as the Magna Mater in Rome) and her consort Attis. Their worship was integrated into the state religion of ancient Rome. The Galli were known for their bright clothing, heavy jewellery, makeup, and bleached, crimped hair. Owing to their self-castration and their adoption of women's clothing, accessories, and makeup, some modern scholars consider them early examples of transgender individuals [6].

The history of the hijra community stretches back to antiquity. The governments of both India (1994) and Pakistan (2009) have recognised hijras as a "third sex", thus granting them the basic civil rights of every citizen. In India, hijras now have the option to identify as a eunuch ("E") on passports and certain government documents. In the Indian subcontinent, hijra are transgender, intersex, or eunuch people who live in communities that follow a kinship system known as the guru-chela system. The term used in Pakistan is khawaja sira, the equivalent of being transgender in the Urdu language [7].

Koekchuch was an extinct gender identity among the Itelmens of Siberia, documented in the late eighteenth and early nineteenth centuries. These individuals, assigned as males at birth, adopted female roles and behaviours. In his work "Description of the Land of Kamchatka," Russian researcher Stepan Krasheninnikov described *koekchuch* as "people of the transformed sex"—a distinct category of men who dressed in women's clothing, performed women's work, and refrained from interacting with men. Krasheninnikov also noted that *koekchuch* served as concubines [8].

Another key term is *nonbinary*. This term emerged in approximately 2011 and was introduced to encompass identities that do not fit within the socially constructed male/female binary. By 2023, "nonbinary" became a breakout term on Google Trends, indicating that searches for the term had increased by more than 5000%. These reflect the broader changes in language, culture, and search engine optimisation (SEO) as well as Google's responsiveness to diverse user needs [9].

Nonbinary individuals are not confused about their gender identity, nor are they following a trend; nonbinary identities have been acknowledged by various cultures and societies worldwide for millennia.

Being nonbinary is distinct from being intersex. Intersex individuals have anatomical or genetic characteristics that do not fit typical male or female categories. Most intersex people identify as either male or female, although some may be nonbinary. Nonbinary individuals are usually not intersex; they are typically born with bodies that conform to conventional male or female definitions, but their innate gender identity is neither male nor female [10].

Overall, as gender identity continues to develop and change, gender identity-affirmative therapeutic approaches that are open to identity exploration with all their associates must be of high importance.

Case example:
Ginny is a 31-year-old who identifies as nonbinary (They/Them). They seek therapy as they would like to explore and understand recent bereavement and relationship problems with their partner.
For the majority of the 46 therapy sessions, Ginny identified as female and lesbian. However, towards the end of therapy, they shared that they felt "nonbinary" was a more accurate identity for them, as they never truly felt comfortable identifying as a woman (wearing dresses, make-up, etc.). This realisation came about during therapy, as they explored what made them "Ginny." This discovery impacted their relationship with their partner, leading them to pursue couples' therapy as well.

Emotional

Sexual orientation refers to an individual's enduring physical, emotional, and/or romantic attraction to others across the spectra of sex/gender. While many people find cohesion between their attractions, their partner's gender, and the labels they use to describe their sexual orientation,

others experience more complex and diverse relationships among these aspects. These individuals often share their unique experiences and the labels that best represent them with a therapist. Women are more likely to report greater overall fluidity in their sexual orientation.

Bisexual orientation is a greater term that may be used by people who occupy many places on the spectrum and/or those who refuse any one specific label on principle. Given that sexual attraction is a continuum, it is no wonder that there is an almost corresponding behavioural continuum of orientation [10].

As mental health professionals, it is important to understand that sexual orientation does not fit into a simple dichotomy of "homosexual" versus "heterosexual." Sexual orientation is diverse and can be expressed across several levels, including gender(s) of attraction or lack thereof (e.g., gynephilic—attracted to femininity; androphilic—attracted to masculinity; amphiphilic—attracted to both or neither). In addition to the gender(s) of attraction, sexual orientation can also refer to the degree of attraction to other people (e.g., aromantic, asexual, etc.) Expressions of sexual orientation are highly diverse and unique to each individual. Sexual orientation has been linked to mental health and cognitive processes, including subjective well-being, attention, and memory. There is an indication that being out about sexual orientation in therapy and being in an affirming therapy setting can have a positive impact on the therapy process. As mental health professionals, it is important to be inclusive of all individuals with various sexual orientations, as these orientations can influence both the mental health symptoms experienced and the therapy process and progress [11, 12].

Spiritual

Gender expression refers to the way an individual publicly presents their gender. This can include their behaviour, clothing, hairstyle, voice, and body language. Gender expression is distinct from gender identity, which is a person's internal sense of their gender (such as being male, female, both, neither, or somewhere along the gender spectrum). Gender expression defines how we present ourselves to the world, peer groups, family systems, etc. Although it is often based on perception, gender expression is anything but binary. To truly understand gender expression, one must also consider gender identity to gain the necessary information to interpret what any given expression might mean for the person displaying it. To that end, therapists committed to affirming rather than pathologising differences must make room for and learn about all possible permutations and combinations of the two and remain aware of the complexity they encounter along the way. Gender expression occurs at many different levels and can be observed in the world at large if one takes the time. It can be tailored to hidden, familial, social, cultural, and religious requirements and discriminatingly reframed to accommodate both personal and situational needs presented by the individual.

Understanding travesty helps illustrate the complexity and individuality of gender identity, sexual orientation, and gender expression, showing that these aspects of identity are not rigid but fluid and personal. A transvestite is a person who dresses as the binary opposite gender expression ("cross-dresses") for any one of many reasons, including relaxation, fun, and sexual gratification (often called a "cross-dresser," and should not be confused with transgender). For example, a person whose sex assigned at birth is male but who chooses to live and present themselves with a feminine gender expression. This can mean that they adopt feminine clothing, hairstyles and behaviours. In regard to gender expression and identity, travesties may not necessarily identify as women but may express feminine gender identity. Their gender expression includes elements typically associated with femininity, but they may have a unique gender identity that does not fit neatly into traditional male or female categories. Their sexual orientation could be towards men, women, both, or neither, independent of their gender identity and expression.

Case example:
Marius (He/Him), a 42-year-old married man with a 3-year-old child, seeks therapy due to marital

issues stemming from his wife's struggle to accept his gender expression. Before our first session, Marius called to ask if he could wear a dress, to which I agreed, encouraging him to wear whatever made him comfortable. He arrived at the session wearing a long summer dress. Marius is a tall, solid man and reports feeling slightly embarrassed about wearing a dress, but mostly, he feels comfortable and happy.

Marius identifies as a man, is sexually attracted to his wife, and occasionally watches straight porn. He began wearing women's clothing after his mother passed away, starting with underwear and gradually incorporating high heels and dresses into his wardrobe. Wearing these clothes makes him feel beautiful and lovable, and it feels like a form of self-care. Throughout 53 therapy sessions, we focused on his relationships with his wife and his late mother, exploring the emotions related to his mother's death and the significance of this loss. Marius eventually came to understand and was able to communicate to his wife that wearing women's clothes is his way of feeling closer to his mother and "continuing bonds" [13].

Stages of Coming Out

There is a stigma associated with LGBTQ+ individuals, and discrimination against nonheteronormative people may occur. The process of revealing sexual orientation or gender identity to friends, family, or colleagues is a gradual or sudden experience. There is an increased risk of depression and anxiety associated with being in the closet in the LGBTQ+ community. Additionally, after they emerge, LGBTQ+ people face significant increases in bullying, experiences of violence, and feelings of alienation. In the long term, however, LGBTQ+ individuals tend to be happier with their lives; they could also have peace of mind, feel less isolated, and suffer from less depression and anxiety. The advantages of coming out can be greatly enhanced by having support and the opportunity to do so while young. In many ways, in society, the workplace, schools, policies and practices have drastically improved [11, 12].

In 1979, a clinical psychologist and sex therapist, Vivienne Cass, developed the Cass theory—a model that describes the developmental process that individuals go through as they consider acquiring their homosexual identity. The model includes lesbian, gay, and bisexual identities [14].

Stage 1: Identity Confusion

In the early stages, individuals usually identify with the heterosexual or gender-conforming majority, often without questioning this identity. However, as they encounter societal norms and expectations, they begin to experience internal conflicts—thoughts, emotions, and physical responses—that challenge their assumed heterosexual identity. These experiences lead them to question whether their true identity aligns with that of the majority.

Stage 2: Identity Comparison

Identity confusion often progresses to a stage of identity comparison rather than resulting in denial or avoidance. In this phase, individuals may begin to feel socially alienated, sensing that they are different from or out of place within the heterosexual or gender-conforming majority. As they become more aware of their differences, they may choose either to embrace their uniqueness or to suppress their true identity to avoid scrutiny and maintain social acceptance.

Stage 3: Identity Tolerance

When individuals enter the identity tolerance stage, they start to acknowledge and accept their connection to the LGBTQ+ community. Instead of concealing their identity, they actively seek out subcultures or communities where they feel they belong. This stage may increase feelings of alienation from the heteronormative majority. Finding supportive subcultures helps ease loneliness and nurtures a sense of community.

Stage 4: Identity Acceptance

Identity acceptance is marked by a deeper connection to the LGBTQ+ community. Individuals in this stage often prefer the company of others who share their identity and begin to distance themselves from cisgender or heterosexual circles. They may start to disclose their identity to close friends, family, or trusted allies, who can offer support and maintain their confidentiality.

Stage 5: Identity Pride

The identity pride stage is characterised by a stronger connection to the LGBTQ+ community. During this phase, individuals often gravitate towards those who share their identity and may begin to distance themselves from cisgender or heterosexual groups. They might choose to reveal their identity to close friends, family, or trusted allies who can provide support and respect their privacy.

Stage 6: Identity Synthesis

The final stage, identity synthesis, involves blending one's LGBTQ+ identity with other facets of one's life, creating a bridge between LGBTQ+ and heterosexual communities. At this stage, individuals come to accept and appreciate supportive cisgender people, recognising that their gender identity is just one element of their broader, multifaceted self. This stage signifies a full-circle moment where individuals embrace a holistic understanding of their identity, acknowledging and integrating its many dimensions [13].

5 Stages of Positive Psychotherapy with LGBTQ+ Clients [15, 16]

Positive Psychotherapy (PPT after Peseschkian) provides a framework for exploring gender identity, sexual orientation, and gender expression in therapeutic settings. While the intervention for members of LGBTQ+ communities generally mirrors that provided to any client, there are specific considerations that must be taken into account.

Acceptance, Observation, and Distancing

In this stage, both the client and therapist take a step back from overwhelming symptoms to gain a new perspective on the client's situation. During this phase of therapy, the psychotherapist must honestly reflect on transference and countertransference. The therapist's cultural background, sexual orientation, gender identity, and expression play a significant role in building a strong therapeutic alliance. Clients often seek acceptance from their therapist and may attempt to conform to what they believe the therapist expects. At this stage, it is crucial for the therapist to be acutely aware of their unconscious biases to effectively challenge the client's internalised homophobia.

Internalised homophobia in psychotherapy refers to the negative feelings and beliefs that might be unconsciously incorporated into the sexual orientation of LGBTQ+ individuals because of societal stigma and discrimination. These could be shown through self-criticism, shame, or even denial of one's sexual identity; this has a significant impact on mental health and well-being. The treatment of internalised homophobia includes the recognition and challenge of negative attitudes, the fostering of self-acceptance, and the development of a positive sense of identity. The therapist will simply provide an environment that is non-judgmental so that clients may explore and, in a safe place, start to tear down the harm of these internalised attitudes.

Questions for therapists to ask themselves:

- Do I privately question the origins of my clients' LGBTQ identities, even if I do not express them aloud?
- Do I believe LGBTQ+ individuals should disclose their identities only if explicitly asked?
- What are my true feelings about legal and religious marriages for gay and lesbian couples?

- How do I feel about LGBTQ+ individuals raising children?
- How comfortable am I discussing the possibility of a 5-year-old being LGBTQ+? Do I assume that children of that age cannot truly understand their sexual orientation or gender identity?

Many therapists hesitate to ask clients about their sexual orientation, and some LGBTQ+ individuals feel disappointed when their therapists do not consider the topic or ask follow-up questions after learning about their sexual orientation [17].

A therapist should use the client's expressed identity and pronouns, and when unsure, it is entirely appropriate to ask the client for clarification. While hearing one's full story may take time and might involve more detailed questioning, trust and rapport building can start during the first conversation by offering affirmation, openness, and acceptance. In initial conversations in which individuals explore their sexual orientation, gender identity and gender expression, it is important to share a general philosophy of cultural humility and ask about what language, cultural, or lifestyle information would be helpful for the therapist to understand and integrate into their care.

Taking Inventory

In *Taking Inventory*, the therapist should assess not only how past experiences affect the present situation but also the stage at which their client is in the coming out process. When assessing *basic conflict*, it is common to find a connection between the secondary capability of orderliness—understood as the perceived way of the world of how things ought to be—and the primary capabilities of contact, love/acceptance, and meaning in life.

Situational Encouragement

In the *Situational Encouragement* stage, the emphasis is on recognising the client's strengths and leveraging their environment as a source of external support. The client must understand that being part of the LGBTQ+ community is just one aspect of their identity. The interventions employed at this stage are similar to those used with cisgender clients.

Verbalisation

Verbalisation is the stage when most of the therapeutic work takes place. Clients' needs vary depending on where they are in the coming out process, which is not linear. Some may have moved from stage 1 to stage 3 and then back to stage 2. During the initial stages, clients are most vulnerable and uncertain about their identity, which can make them susceptible to conversion or reparative therapy in an attempt to feel "normal." As they begin to recognise themselves as part of the LGBTQ+ community, they may experience heightened anxiety, self-consciousness, and sensitivity to negative evaluations, whether perceived or real. In these early stages, clients face the crucial decision of whether to accept or deny this aspect of themselves. They often compare themselves to others to determine if they can tolerate the coming out process. Therapists need to provide a safe space where clients can explore their identity and feel empowered in their journey [17].

There is a misconception among therapists and the general public that simply because a person is openly gay, lesbian, bisexual, or transgender, that person is well informed of LGBTQ+ culture. This is far from accurate. Given the marginalisation of LGBTQ+ culture and history, many people who are gay or transgender are as uninformed about their own culture as heterosexuals. Many questions and issues arise when working with an queer person, and a lack of understanding of these issues can widen the gap of misunderstanding between client and therapist.

As therapists, we should not underestimate the culture shock our clients might experience when they first step into the LGBTQ+ community. Having a friend who is part of the community might help our clients safely navigate the rules of

interaction. Equally important is having a gay-friendly and affirmative therapist who offers a nonjudgmental reflection space and gently challenges internalised homophobia.

Case example:
Deniz, a 25-year-old student of Asian descent, sought therapy to better understand his sexual orientation. After a drunk night, he ends up kissing and engaging in a sexual act with one of his friends, which leaves him confused. Coming from a traditional family in which homosexuality is not accepted, Deniz is eager yet anxious to explore his sexual identity. He decides to visit a gay club to see if he might identify with the LGBTQ+ community. Despite my caution about potential culture shock and my suggestion to go with one of his openly gay friends, Deniz goes alone. He is pursued by a man, whom he perceives as aggressive. This leads him to conclude that he does not want to be part of the LGBTQ+ community. Following this experience, Deniz stops attending therapy.
After approximately six months, Deniz returned to therapy, having tried reparative therapy, which only increased his anxiety and depression. This time, he is committed to focusing on self-care and taking his journey of self-discovery more slowly.

In the later stages of coming out, clients tend to have greater awareness and acceptance of their LGBTQ+ identity. The focus of therapy then shifts to fostering social and intimate relationships within the LGBTQ+ community. Therapeutic work during these stages often involves bringing to light and addressing dysfunctional behaviour patterns, unrealistic expectations, developmental injuries, and repressed traumatic memories.

It is especially important to address *intersectionality* when working with LGBTQ+ clients. The term was first coined by Kimberlé Crenshaw in 1989 and was used to describe how different forms of discrimination interact simultaneously and produce further inequalities [18]. This is particularly useful when thinking about mental health experiences. Not everyone in the LGBTQ+ community will experience mental health difficulties, but for those who do, their experiences may not be fully understood unless other aspects of their lives are taken into account [19].

Case example:
Jocelyn (she/her), a 54-year-old Christian woman, is married with two adult children. She came to therapy to explore difficulties in her relationship with her husband and some confusing feelings she had been experiencing toward a new female friend. She has a strong support system within her family and her church community.
During therapy, Jocelyn reflected on the fact that she had never felt a sense of belonging in her marriage and had never been sexually attracted to her husband or any other man. She attributed these feelings to her strong Christian upbringing, which emphasised that sex is solely for procreation. After her children moved out and started their own families, Jocelyn found herself with more time to focus on her own needs and began forming new friendships outside of the church. She met Maria in a photography course and felt an immediate sexual attraction to her. Maria, a 48-year-old openly lesbian woman, reciprocated these feelings. Jocelyn felt overwhelmed, anxious, and disgusted by her sexual attraction, wishing she could return to feeling "normal."
Throughout therapy, Jocelyn grappled with the conflict between her feelings for Maria and her Christian values and community. As positive psychotherapists, we can recognise the key conflict between honesty/openness and politeness/courtesy.
Jocelyn ultimately concluded that there was no compromise and decided to remain "in the closet". She feared that coming out as bisexual or lesbian would lead to being rejected by her church community, losing her family relationships, and even her children.
We discussed the potential consequences of her decision, including the possibility of ongoing anxiety and the risk of developing more severe mental health issues in the future. Jocelyn chose to join a Christian group that supports LGBTQ individuals in living chaste lives. As a therapist, I felt torn—on the one hand, feeling that I had failed to help her live an authentic, unashamed life but on the other hand, supporting her in making a decision that respected all aspects of her intersectional identities.

Broadening of the Goals

The *Broadening of the Goals* part of therapy with LGBTQ+ clients should focus on integrating the new identity with other identities bridging the divide between new and old. This part of therapy should coincide with stage 6 of the coming out process, a full-circle experience when clients can embrace and unify all aspects of who they are.

Summary

This chapter explores the evolving role of psychotherapy in supporting LGBTQ+ individuals, emphasising the need for cultural sensitivity and inclusivity. It outlines the complexities of providing affirmative care that respects the intersections of gender identity, gender expression, and sexual orientation.

Key Points

1. Cultural Sensitivity and Inclusivity: Modern psychotherapy must adapt to the changing societal acceptance of diverse identities, acknowledging and addressing the unique needs of LGBTQ+ clients with sensitivity and humility.
2. Understanding Labels: The use of labels such as LGBTQ+ (lesbian, gay, bisexual, transgender, queer/questioning, and others) helps validate and affirm clients' identities but can also lead to oversimplification and pressure. Therapists should balance the use of labels, allowing clients to define their own identities and respect their preferences.
3. Concepts of identity:
   - Sex Assigned at Birth: Refers to physical characteristics categorised as male or female.
   - Gender Identity: An individual's internal sense of their gender, which may align with or differ from their assigned sex at birth.
   - Sexual Orientation: Describes the spectrum of emotional, physical, and romantic attraction to others.
   - Gender Expression: The outward presentation of gender, which can vary widely and may not align with societal norms.
4. Stages of Coming Out: Cass's model outlines stages from initial identity confusion to the integration of LGBTQ+ identity with other aspects of self.
5. Positive Psychotherapy Therapeutic Approach:
   - Observation and Distancing: Involves stepping back to understand the client's perspective, considering both client and therapist biases.
   - Taking Inventory: Assesses the client's coming out stage and past experiences.
   - Situational Encouragement: Focus on leveraging strengths and support systems.
   - Verbalisation: Provides a safe space for clients to explore and affirm their identities.
   - Broadening of the goals: The client is invited to reflect on their therapeutic journey and embrace all aspects of who they are.
6. Intersectionality: Therapists must consider how multiple forms of discrimination and identity factors interact with and affect clients' mental health. A holistic approach integrates diverse aspects of identity and supports clients in bridging gaps between new and old parts of themselves.

Therapists working with LGBTQ+ individuals must consider several ethical points to ensure competent and inclusive care. One crucial aspect is the need for cultural competence, which involves understanding and respecting the cultural backgrounds and experiences of LGBTQ+ clients. This includes being aware of societal standards, such as oppression and discrimination, and helping clients address internalised homophobia and explore feelings related to these issues. Furthermore, it is essential for therapists to address countertransference that may occur due to biases and to refrain from implying that LGBTQ+ identities are pathological. Even if a therapist does not personally agree with or fully understand a client's life choices, preferred identity, or sexual orientation, they must still offer respect. By actively combating biases and understanding the client's narrative and experiences within their social context, therapists can effectively connect with LGBTQ+ clients in a meaningful and genuine way. These ethical

responsibilities are crucial for therapists providing competent and inclusive care to LGBTQ+ individuals.

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47 Pathways to Trauma Healing Using Positive Psychotherapy: Experience with Severely Traumatized Adult War Survivors

Solomon Abebe Woldemariam

Introduction

We are living in an era where the effects of trauma, particularly from war and conflict, are deeply entrenched in our global consciousness. Ongoing conflicts, internal and external displacement, and the aftermath of violence have left individuals and entire communities struggling with long-term emotional and psychological wounds. War-related trauma affects not only people directly involved in the war but also ripples through generations, affecting mental health, social relationships, and spiritual health. As we become more aware of the severe repercussions of such traumatic experiences, there is a growing emphasis on addressing the needs of those affected through therapy, support systems, and initiatives aimed at healing and rebuilding lives shattered by conflict.

Severe traumatic experience resulting from war imposes deep and enduring damage specifically to individuals and communities in general. The extreme violence, death, and displacement experienced during conflict lead to significant psychological wounds. Individuals exposed to war are at high risk for developing mental health problems such as post-traumatic stress disorder (PTSD), anxiety, and depression. Studies show that people living in war-affected areas are more likely vulnerable to mental health problems due to their direct exposure with violence and loss [1].

In addition, war disturbs social and cultural ties within communities. The violence and instability destabilize social bondage and support systems that are indispensable for healthy mental functioning. As a result, the capacity of trust among community members becomes weak, and existing tensions are often exacerbated, leading to further disintegration. This cracking of social bondage prevents significant recovery and upgrading efforts, as lack of trust makes it difficult for individuals and communities to come together and get recovery [2].

The psychosocial challenge caused by war serves as a catalyst to bring additional layers of traumatic experience. The destruction of human power, essential services, and infrastructures mainly healthcare lead the population into the quagmire of poverty. Furthermore, Internal and external displacement exacerbates feelings of loss and hopelessness among affected populations [3]. Positive Psychotherapy, as developed by Nossrat Peseschkian (Peseschkian, 1980, 1987, offers a structured approach to intervene in traumatic experiences.

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_47

Definition of Key Terms

Pathway

A pathway is a route or course of action that leads to a specific goal or outcome, often involving a series of steps or processes.

Trauma

Trauma, from a psychodynamic perspective, is understood as a profound emotional response to an event or series of events that overwhelms an individual's capacity to cope and integrate the experience. Trauma is seen as disrupting the individual's internal world, leading to fragmented and repressed memories that resurface through symptoms such as anxiety, depression, or dissociation. These symptoms are manifestations of unresolved conflicts and intrapsychic struggles that stem from the traumatic event. This approach emphasizes the importance of uncovering and processing these repressed elements to achieve psychological healing and integration [4, 5].

Severe

In the context of trauma, "severe" describes experiences or symptoms that are intensely distressing and significantly disrupt an individual's ability to function in daily life, often requiring specialized and intensive therapeutic intervention.

Survivors

Survivors are individuals who have experienced and endured traumatic events, often engaging in a process of recovery and healing to overcome the long-term effects of those experiences.

Applications

The aim of PPT is to help clients see their abilities, strengths, resources, and potentials behind their traumatic experience. Trauma healing process is based on the three main principles: the principle of hope, the principle of balance, and the principle of consultation (Peseschkian, 1980, 1987). The principle of hope helps the client to see the meaning and purpose of trauma. Then, trauma will be interpreted positively. Using the principle of balance the client will get an opportunity to deeply understand body/health, achievement/work, contact/relationships, and future/purpose/meaning of life in connection with traumatic experience. The five stages of positive psychotherapy (principle of consultation) represent a concept in which therapy and self-help are closely interrelated. The patient and the family are getting informed together about the illness and the individual solution to it. The five stages of positive psychotherapy include Distancing, Inventory, Situational encouragement, Verbalization, and Broadening of goals. Detailed application procedures will be presented in the next section in detail.

Distancing

This step follows several technical procedures to facilitate the healing process. This part contains vital elements including preparation, history collection, establishing therapeutic alliance, and screening, assessing window of tolerance, stabilization, and identifying actual conflict. All the aforementioned elements play a great role in creating an organized and effective healing process.

Preparation

Preparation especially in the Trauma Healing process is a critical element that considers logistical and cultural considerations. Initially, it is mandatory to ensure and organize a safe therapy environment. The therapist must be sure that clients feel secure. In addition, a deep understanding of the environment, specifically language, culture, social norms, religion, and spiritual practice is helpful for the healing process – particularly their tendencies toward fusion, differentiation, and detachment, which helps tailor the healing effectively. If the therapist is not aware of his client's language, arranging a translator is

critical for clear communication. Additionally, during the trauma healing process the therapist should be skillful in utilizing advanced grounding techniques and other self-regulation techniques in order to manage emotional distress and abreactions during the healing process. Otherwise, the therapist needs professionals who assist him in self-regulation.

History Collection

History collection during trauma healing involves clinical history, personal history, childhood history, school life history, relationship, and social history. It is also very important to identify major life events that happened in the past 5–10 years.

Establishing a Therapeutic Alliance

During trauma intervention establishing a therapeutic alliance is an essential element for achieving an effective trauma healing process. Establishing a therapeutic alliance in connection with the trauma healing process demands creative capacity, empathy, active listening, trust, collaboration, and maintaining professional boundaries. It also depends on the therapist's ability to create safe, non-judgmental therapeutic sessions. For a relatively effective and sustainable outcome the client should feel security and support. Stronger therapeutic relation heightens client's engagement and further accelerates the healing process, promotes emotional openness, and paves the way for deeper exploration of other deep rooted traumatic memories. According to various research literatures, therapeutic alliance is highly correlated with better treatment outcomes [6].

Screening

During trauma intervention rapid screening is important to understand the level of disturbance, stress, or PTSD level. Accordingly, initially the therapist should collect baseline data using the following tools: Adverse Childhood Experience (ACE) questionnaire, together with Modified PTSD symptom Scale (MPSS-SR), PTSD Checklist for DSM-5 (PCL-5) or any other relevant screening tools. The therapist can measure PTSD in three phases mainly pre-test (Before the therapy) and mid-test assessments (in the middle of therapy sessions) and post-test assessment (during closure session of the therapy). The ACE questionnaire helps identify early life adversities that contribute to long-term mental and physical health risks, offering a comprehensive understanding of each participant's trauma history. Various research showed that using screening tools can be highly effective to understand how the healing process reduces trauma-related signs and advance overall functioning (Felitti et al., 1998).

Assessing the Window of Tolerance

Checking the level of the client's window of tolerance during the trauma healing process is fundamental for guaranteeing clients remain in a convenient emotional and physiological state. It is difficult to start and continue the healing process if clients are not in the optimal level of window tolerance. The window of tolerance, as it is introduced by Siegel [7], symbolizes the optimal range where clients can experience and regulate emotions devoid of hyperarousal (e.g., anxiety, panic) or hypoarousal (e.g., numbness, dissociation). Therapists should assess the level of window of tolerance through carefully spotting clients' verbal and non-verbal signals, such as variations in body language, speech patterns, or emotional responses. Consistent monitoring is helpful for the therapist to ensure that clients stay emotionally engaged and can safely process trauma within the therapeutic setting [8]. There are several techniques that will help clients back into their window of tolerance—indicated in the stabilization section below.

Stabilization

Stabilization is the backbone of the trauma healing process. Without stabilization techniques it will be difficult to imagine a relatively sustainable change in the healing process. Teaching client's stabilization during the trauma healing process comprises preparing clients with proven tools and techniques for managing intense emotions and reclaiming a sense of control during distress. Stabilization should be done in the first phase of trauma healing, focused on supporting

clients to develop basic emotional regulation skills before delving into traumatic memories. Grounding exercises are commonly recommended techniques of stabilization, which anchor clients to be in the moment-by-moment awareness. Using different types of **breathing techniques** recommended for calming the nervous system and body awareness. **Psychoeducation** is also an indispensable component and it will create a chance for clients to learn about the body's stress response and recognize warning signs of dysregulation. By learning and regularly practicing the above-mentioned stabilization techniques, clients can stay within their window of tolerance. Stabilization has a power to build resilient behavior and enables clients to cope with present trauma-triggering factors both during and outside of healing sessions [9]. These techniques have been used with severely traumatized war survivors including: Diaphragmatic Breathing, storytelling, Interval Training, relaxation training (Peseschkian, 1987), and body scanning hypnotic relaxation developed and implemented by Solomon Abebe.

Diaphragmatic Breathing Diaphragmatic breathing in some literature mentioned as a deep breathing or belly breathing. The mechanism behind diaphragmatic is to maximize blood oxygenation by engaging the diaphragm. This method can be particularly useful for trauma survivors because it activates the parasympathetic nervous system which is helpful to calm the body and reduce the stress level. The therapist can follow the following procedure to clients about Diaphragmatic breathing: Find a comfortable position; Place one hand on your chest and the other on your abdomen.; Inhale deeply through your nose, ensuring that the hand on your abdomen rises while the one on your chest stays relatively still; Exhale slowly through your mouth, feeling the hand on your abdomen lower as air leaves your lungs; Repeat this process, focusing on slow, steady breaths. Even though diaphragmatic breathing is generally safe for most people, clients with respiratory conditions, cardiovascular issues, severe anxiety or panic disorders and clients recovering from recent surgery should consult with a healthcare provider before practicing it regularly.

Storytelling N. Peseschkian compiled around 100 oriental stories for use in therapy [10]. The therapist should select convenient oriental stories for the healing process. It is also possible to use indigenous stories that can bring change in perspective. The oriental stories served as mirror model, mediator, repository effect, regression, and change perspective of trauma survivors.

Interval Training by N. Peseschkian

Interval Training as introduced by Nossrat Peseschkian [11] is a therapeutic method mixed with physical activity with psychological insights. The method is useful in integrating both body and mind in the healing process. In this method, physical activities like walking, running, or any form of exercise are alternated with periods of rest or lighter activity. This cycle of intensity and recovery improves physical, mental, emotional, social, and spiritual resilience.

Peseschkian strongly believed that integrating physical exercises into psychotherapy could help clients to manage their stress and emotional disturbances. Therefore, the body movement in interval training promotes emotional regulation, as the physical activity helps release tension while the mental focus initiates mindfulness. This method linked with Peseschkian's strong view of balancing mental and physical health to enhance overall well-being.

Using Interval training in group trauma healing sessions help clients to process and release accumulated stress safely. The author of this chapter modified Peseschkian's original Interval Training for decreasing stacked traumatic memories for severely trauma survivors. The modified interval training version contains nine procedures. The modified versions were repeatedly administered among severely traumatized war survivors and found to be effective for the trauma healing process.

According to the revised version of interval training, the first procedure includes Identifying

disturbing or traumatic point of disturbance; and measuring point of disturbance in a scale of 0–10 where 0 indicates no disturbance and 10 indicates highest level of disturbance. The therapist should focus only on one point of disturbance. After finishing one disturbance it is possible to start working with the next disturbance.

The second procedure includes playing peaceful music as deemed necessary depending on the interest of clients, Breathe deeply while lifting both shoulders and arms and breathe out while letting the shoulders and arms fall. This activity should be done 20 times.

The third procedure incorporates: Move arms forward, in a circular motion; afterwards move arms backward, making another circle; and Remain standing all the while. This activity should be done 20 times.

The fourth procedure includes a confrontation phase with disturbance. It has the following activities: Bring point of disturbance in front of you by imagination; Adopt a boxer stance, with the knees straight; Swivel the hips; First boxing with left hand and holding right in back, then vice versa. This activity should be done 20 times. Then the therapist should help clients to measure the point of disturbance again on a scale of 0–10 where 0 indicates no disturbance and 10 indicates highest level of disturbance. Check if there is any change on the client's disturbance. You can make it a 5–10 cycle until the level of disturbance dwindles.

The fifth procedure embraces: Bending forward; Standing with legs shoulder width apart, bend forward with knees straight, touching the tip of the right and then left foot; and Stay bent over. This activity should be done 20 times. Measure point of disturbance in a scale of 0–10 where 0 indicates no disturbance and 10 indicates highest level of disturbance.

The sixth procedure is to continue with Bending forward again, like the fourth Exercise, but after touching the tip of one foot, stand and stretch vigorously. This activity should be done 20 times. The therapist should help the client to measure the point of disturbance in a scale of 0–10 where 0 indicates no disturbance and 10 indicates highest level of disturbance.

The seventh procedure is to continue with Exercise. It includes the following activities: Knee bends, Stretch your arms straight ahead while counting, One- Crouch down, Two- stand up, Three- Stretch your arms out sideways, Four- Rest your arms by your side again. This activity should be done 20 times. The therapist should help the client to measure the point of disturbance in a scale of 0–10 where 0 indicates no disturbance and 10 indicates highest level of disturbance.

The eighth procedure is interval training. By this time clients will be warmed up. Now, the client is ready to begin the interval training itself. First help the client to run in place in a stretched out position, moving legs and feet softly forward and backward. In the process, the arms move alternatingly up and down, resting the palm of the lower hand on the thigh. Play fast-paced music in the background as deemed necessary. The therapist should help the client to measure the point of disturbance in a scale of 0–10 where 0 indicates no disturbance and 10 indicates highest level of disturbance.

The ninth procedure focuses on pausing, swinging arms gently back and forth, walking around for 2 or 3 min, audibly inhaling through the nose and exhaling through the mouth as you go. The therapist should help the client to measure the point of disturbance in a scale of 0–10 where 0 indicates no disturbance and 10 indicates highest level of disturbance. Play soft background music as deemed necessary.

Body Scanning Hypnotic Relaxation Script as Developed by Solomon Abebe

Relax…

You can sit or lie down in a comfortable position. Make sure that you are in a comfortable position. This is body scanning hypnotic relaxation. Do some breathing exercises. Breathe in deeply and breathe out slowly. Imagine your breathing in has a healing power and it can create a healing energy. When you breathe out imagine you are removing all unwanted feelings, tightness, and inconvenience and body sensations from your body.

Trust on what you are doing. In case you have different, unnecessary or irrelevant thoughts that

come up during the process don't judge yourself or the situation. Just leave it and slowly back to your breathing.

Relax…

Now, imagine your foot. Your foot served you several years. Feel the soul inside your foot. Your foot is a very important part of your body. Your foot has a connection with other parts of the body. Try to listen, feel and touch (if possible) your foot. **Relax…**If you feel uncomfortable, pain or tightness spot the area. Now, focusing on your foot and sole, give it healing energy by taking a deep breath. When you breathe out you can remove all your uncomfortable feeling, pain or tightness from your foot areas. You can take three cycles of breathing.

Relax…

Imagine now you have created an amazing healing energy inside your foot. This energy is diversified itself and moving slowly to our ankle. This energy directs itself at the shape of your ankle, resonates and vibrates around. If you feel uncomfortable, pain or tightness spot the area. Now, focusing on your Ankle, give it healing energy by taking a deep breath. When you breathe out you can remove all your uncomfortable feeling, pain or tightness from your Ankle areas. You can take three cycles of breathing.

Relax…

Now, the healing energy, which is created inside our foot, is expanding and moving upward towards our legs simultaneously. This energy touches leg parts mainly fibula and tibia and deep the bone marrow and calf muscles. If you feel uncomfortable, pain or tightness spot the area. Now, focusing on your legs, give it healing energy by taking a deep breath. When you breathe out you can remove all your uncomfortable feelings, pain or tightness from your foot areas. You can take three cycles of breathing.

Now the healing energy diversified its force, rapidly moving upward to your knee parts, mainly tibia (shin bone), and patella (kneecap), and ligament. If you feel uncomfortable, pain or tightness spot the area. Now, focusing on your Knee, give it healing energy by taking a deep breath. When you breathe out you can remove all your uncomfortable feelings, pain or tightness from your knee areas. You can take three cycles of breathing.

Relax…

The energy is further stretching its power to the femur (thigh bone). It touches the pectineus, sartorius, and quadriceps femoris and thigh muscles. If you feel uncomfortable, pain or tightness spot the area. Now, focusing on your thigh area, give it healing energy by taking a deep breath. When you breathe out you can remove all your uncomfortable feelings, pain or tightness from your thigh bone areas. You can take three cycles of breathing.

Relax…

The healing energy reached to the hip and continued with various muscles and blood supplies, lymphatic structures, nerves, and organs, including the intestines, urinary bladder, and internal sex organs.

If you feel uncomfortable, pain or tightness spot the area. Now, focusing on your foot and sole, give it healing energy by taking a deep breath. When you breathe out you can remove all your uncomfortable feeling, pain or tightness from your hip areas. You can take three cycles of breathing.

Relax…

Now, the healing energy moves to the abdominal organ and touches all abdominal organs, mainly the stomach, duodenum (first part), jejunum, ileum, liver, gallbladder, the tail of the pancreas, spleen, and the transverse colon. Relax…If you feel uncomfortable, pain or tightness spot the area. Now, focusing on your abdominal organs, give healing energy by taking a deep breath. When you breathe out you can remove all your uncomfortable feeling, pain or tightness from your abdominal organs. You can take three cycles of breathing.

Relax…

The healing energy is heading to the chest cavity areas mainly heart, esophagus, trachea, lungs, and thoracic diaphragm. If you feel uncomfortable, pain or tightness spot the area. Now, focusing on your chest cavity, give it healing energy by taking a deep breath. When you breathe out you can remove all your uncomfort-

able feeling, pain or tightness from your chest areas. You can take three cycles of breathing.

Relax…

The energy rapidly moves to the shoulder, upper arm, lowers arms and fingers. Further to nape and back bone. If you feel uncomfortable, pain or tightness spot the area. Now, focusing on your shoulder, upper arm, lower arm, nape and the backbone areas, give healing energy by taking a deep breath. When you breathe out you can remove all your uncomfortable feelings, pain or tightness from this area. You can take three cycles of breathing.

Relax…

Finally this healing energy is moving upward to our head parts, mainly the ear, the eye, the nose and sinuses, the salivary glands, and the oral cavity. Further scanning our brain parts mainly cerebrum, cerebellum, and brainstem. If you feel uncomfortable, pain or tightness spot the area. Now, focusing on your head parts and brain, give healing energy by taking a deep breath. When you breathe out you can remove all your uncomfortable feeling, pain or tightness, negative thoughts from your head and brain areas. You can take three cycles of breathing.

Relax…

Breathe in…. Breathe out….

I'm going to count the numbers from one to five, and at the count of five, you will be wide awake, feeling already wonderfully energized and ready to allow these suggestions to work for you. One, two, three, coming slowly back. Four—eyelids beginning to flutter, and five—eyes open, wide awake, mind and body returning to normality.

Identify Actual Conflict

Identifying actual conflicts is a pivotal role to understand the root causes of traumatic experience. It focuses on understanding the collision between a need and reality. Clients may experience several actual conflicts. However, it is essential to support them in prioritizing the dominant actual conflict and it is good to help clients to measure the level of disturbance on a scale from 0 (no disturbance at all) to 10 (maximum level of disturbance). Measuring actual conflict helps both the therapist and client in identifying the most pressing issues and allows for more targeted and effective healing [5] process (Table 48.1).

Table 48.1 Matrix for Measuring traumatic actual conflicts and level of disturbance

No	Perceived actual conflicts	Level of disturbance 0 1 2 3 4 5 6 7 8 9 10
1		
2		
3		
4		
5		
6		
7		
8		
9		
10		

The process examining the tension between clients' need and reality is necessary for ensuring self-awareness and enabling the client to be at the present moment. The result of the examination helps as a signal in order to uncover the underlying disharmony that ignites internal conflicts. In addition, clients can have insight and it will open pathways to heal unresolved issues at the conscious mind.

Understanding the location of actual conflict is important. The location can be dynamically manifesting itself in one of four areas of life mainly: body, work/achievement, contact, and spirituality (Peseschkian, 1980). Recognizing the location of the actual conflict provides valued insight for understanding the focus of the healing process.

After understanding the locus of Actual conflict the next task will be identifying the contents of actual conflict. Most often, actual conflict stems from the internal needs of clients and clients may not fully recognize or articulate it directly. The hidden internal needs of the client are usually revealed through the client's statements and behaviors. The actual conflicts may revolve around the client's learned (secondary capacities) values. These values may include punctuality, cleanliness, orderliness, obedience, courtesy, honesty, faithfulness, justice, diligence, thrift, reliability, and conscientiousness. It is valuable for the therapist to mindfully listen to

his clients' view to identify underlying needs. We can use the relation model "I, You, We & Primary We" model in order to deeply understand the importance of the contents of actual conflict. Simply, it is possible to ask the importance of the content for the client, partner/family, colleagues/friends, and for the world in general (Fig. 48.1)

Often, reactions to actual conflict often manifest itself in the body, work, contact, and futurity. In the body area clients may react through tension, headaches, or gastrointestinal issues. In the work/achievement area, clients may react with poor performance, job dissatisfaction, or stressed professional relationships. In social contact, clients may react through difficulty in communication, frequent conflict. In the spirituality/futurity area clients may have reactions like strong doubt about the future, procrastination, and a lack of direction (Table 48.2).

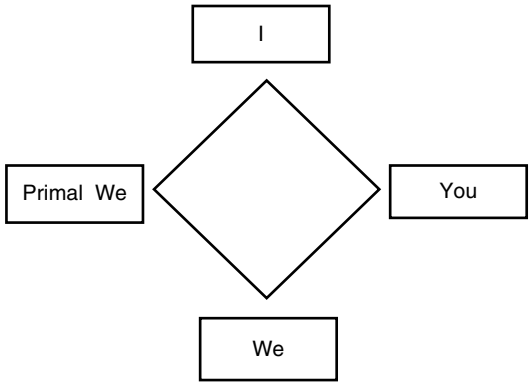


Fig. 48.1 Relation model to identify the importance of Actual conflict content

Inventory

This is a deep understanding phase of clients in trauma healing sessions. It is highly recommended to do the following pillars of activities, mainly understanding clients by using the balance model in four major areas of life: Body, Contact, Achievement, Spirituality. In addition, therapists should support clients to map out traumatic experience clients starting from childhood in order to get to the touchstone trauma. Followed by helping clients to understand basic conflict, identify Key Conflict, and be aware of Internal conflict.

Understanding a Client's Status in Four Major Areas of Life

Body, Contact, Achievement, Spirituality. To assess the body area, it is crucial to explore any health issues, nutrition and dietary habits, physical activity, sleep quality and sleep habits, daily rest and relaxation, bodily wellbeing practices, substance use, body image and perception, and attitude towards sex.

To understand achievement dimensions, we may need to assess a client's job satisfaction, job security and financial stability, work hours and

Table 48.2 Secondary capacity matrix helps to understand contents of actual conflict

MINIMUM If it is at the minimum level which word describes best for you?	SECONDARY CAPACITIES UNDER OPTIMAL LEVEL	MAXIMUM If it is at the maximum level which word describes best for you?
	PUNCTUALITY	
	CLEANLINESS	
	ORDERLINESS	
	OBEDIENCE	
	COURTESY	
	HONESTY/CANDOR	
	FAITHFULNESS	
	JUSTICE	
	DILIGENCE/ACHIEVEMENT	
	THRIFT	
	RELIABILITY/PRECISION	
	CONSCIENTIOUSNESS	

workload, work-related stress and fear of failure, response to performance criticism, handling downtime, career aspirations and preferred work, parental appraisal, career choice.

It is also important having adequate information in the dynamics of social contact. It is recommended to examine clients' partnership satisfaction, reasons for dissatisfaction, partners' compassion, social preferences in relationship, time spent with others, parental relationship and confidence, challenges from family and social contacts, social and emotional fulfillment, and impact of external opinions.

Another important issue is having ample information about the dynamics of futurity or spirituality. Therefore, it will facilitate the healing process exploring clients' thoughts and reflection, contemplation of mortality, beliefs about afterlife, search for meaning, religious and philosophical background, valuing life and health, future aspirations, urgent needs for change, life without fear or depression, and greatest desire.

Mapping Traumatic Experience

Mapping out traumatic experiences based on age category is crucial. It will help the therapist to understand detailed traumatic experiences and enable addressing touchstone trauma (Table 48.3).

Understanding of Basic Conflict

Basic conflict focuses on understanding dysfunctional stored family concepts or philosophy. It is also useful to understand the location and contents of basic conflicts. We can use the PPT model dimension model to understand the dynamics of basic conflict (Fig. 48.2).

In order to understand the contents of the basic conflicts, we can use the primary capacities as a reference. These are basic capacities that every human being should get. However, deficiency of these capacities is very common among clients. Therefore, love connected values (Primary capacities) can feed the basic conflict. It includes time, patience, and contact, love/acceptance, sexuality/tenderness, trust, confidence, doubt, hope, faith, modeling and unity (Table 48.4).

Key Conflict

The tension between Politeness which is considered as exercising the power of "Yes" and Honesty reflected as exercising the power of "No" are sources for the key conflict [11]. Politeness, usually driven by the need to maintain interpersonal relationships even when it contradicts with one's true desires. This habitual tendency is highly rooted in the desire to avoid conflict leading to gaining approval and also discomfort. On the other hand, honesty needs the courage to assert restrictions and speak openly at the risk of creating friction or rejection in the interpersonal relationship. This conflict between societal expectations and personal reality can generate inner turmoil as clients tackle when to prioritize politeness over honesty and vice versa [12]. During the trauma healing process understanding the key conflict fosters positive outcomes.

Table 48.3 Matrix for mapping traumatic experience

Mapping traumatic experience Based on age category	Rate the level of disturbance 0–10
Early childhood	
Middle childhood	
Adolescence	
Early adulthood	
Middle adulthood	
Late adulthood	

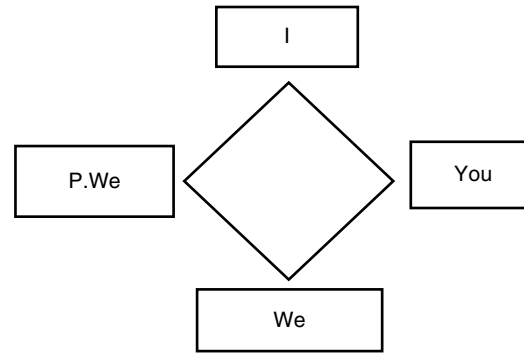


Fig. 48.2 Model dimension for understanding basic conflict

Table 48.4 Primary capacities for identifying contents of basic conflict

MINIMUM If it is at the minimum level which word describes best for you?	PRIMARY CAPACITIES UNDER OPTIMAL LEVEL	MAXIMUM If it is at the maximum level which word describes best for you?
	TIME	
	PATIENCE	
	CONTACT	
	LOVE/ACCEPTANCE	
	SEXUALITY/ TENDERNESS	
	TRUST	
	CONFIDENCE	
	DOUBT	
	HOPE	
	FAITH	
	MODELING	
	UNITY	

Internal Conflict

Internal conflict will be complicated when tangled with traumatic experiences. Usually trauma magnifies unresolved compromised capacities developed at early stages by keeping them unconscious. Trauma can trigger or worsen inner conflicts. Unresolved basic conflicts related to early emotional wounds may reappear when confronted with actual conflicts triggered by trauma. The fusion of actual conflict and basic conflict creates a powerful internal barrier, making it difficult for clients to process trauma effectively. In addition, the combination of past and present conflicts can overwhelm coping mechanisms and hamper recovery [12, 13].

For example, internal conflict connected with trauma can be seen in a person who experienced emotional neglect in childhood, creating a basic conflict of feeling unworthy of love. As an adult, they might face an actual conflict in a relationship if their needs are ignored, reinforcing this belief. Traumatic incidents reactivate unresolved internal conflict and make healing difficult. Childhood neglect, present challenges, and inability to express openly creates psychological barriers and further deepening the impact of the trauma and deterring emotional recovery.

Situational Encouragement

For the purpose of trauma healing situational encouragement highlights four key elements: verification, which ensures the client feelings and experiences are acknowledged; resource mapping, which provide the tools or support needed to cope with trauma; self-regulation resources that helps client to manage emotions and behaviors; and encouragement, which fosters motivation and confidence to move forward in the healing process. Together, all the above-mentioned elements have the power to create a supportive environment that empowers clients to navigate traumatic battles effectively.

Verification

In trauma healing verification is the benchmark to understand if, and how, the actual conflict is connected with the unconscious basic conflict. It involves exploring how excess demand for learned capacities developed in order to get love or acceptance. Therefore, at the stage of verification, it is necessary to reach a common ground between the therapist and the client, recognizing to what extent childhood development patterns impact present conflicts and ensuring that client

capacity to get acceptance is not overwhelmed by excessive demands of learned capacity.

Resource Mapping

Resource mapping is a highly encouraging tool that has the potential to identify and recognize valuable resources for facilitating the healing process. The Balance Model provides a structured way to explore resources around four key dimensions. "I" focuses on resources linked with clients' capabilities and self-resources, "You" resources obtained from contact or relationships", "We" concentrates on resources gained from social and community support, and "Primary WE" resources connected with spiritual power or any other philosophical stands (Fig. 48.3).

Using Self-Regulation Resources

Resources also served as a tool for self-regulation which involves stabilizing emotional disturbances effectively. For instance, if a client has identified mindfulness practices or a calming environment as valuable resources, they can be guided to use these during moments of emotional distress. By incorporating these resources into their daily routines or specific situations of emotional disturbance, clients can practice meditation or other self-regulation techniques that help them stabilize their emotions and reduce stress. This approach not only empowers clients to take control of their emotional well-being but also reinforces their ability to manage future challenges by drawing on the strengths and supports they have recognized.

Encouragement

Encouragement is a critical aspect of trauma therapy, as it involves recognizing and reinforcing clients' strengths and capabilities observed during their therapeutic journey. When working with trauma survivors, therapists focus on highlighting positive attributes and progress, such as resilience, courage, and coping skills, that clients demonstrate despite their traumatic experiences. For example, if a client shows significant progress in managing their trauma-related symptoms or develops effective coping strategies, the therapist provides positive reinforcement and encouragement. This not only acknowledges the client's efforts but also helps build their confidence and self-efficacy, essential for overcoming trauma. Encouragement helps clients feel validated and supported which can empower them to continue engaging in their healing process and apply their strengths to address and manage their trauma effectively. By fostering a sense of competence and self-worth, encouragement can play a pivotal role in the recovery and resilience of trauma survivors.

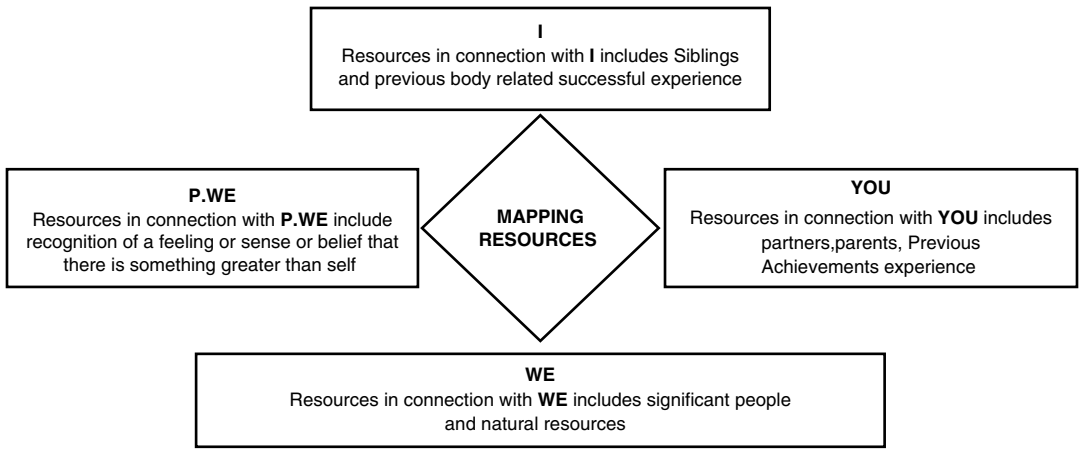


Fig. 48.3 Resource mapping model

Verbalization

Verbalization in trauma therapy is an assertive communication stage that helps clients navigate their responses to traumatic situations by clearly identifying what they can and cannot change. This process supports clients in recognizing uncontrollable aspects of their situation and focusing on areas where they can exert influence or make improvements. For example, clients might discern between immutable past events and present behaviors they can adjust to better manage their trauma [14].

Additionally, clients are encouraged to explore and develop new capabilities to enhance their productivity and resilience in the face of trauma. This involves reflecting on their current perception of the situation to maximize benefits and deciding what actions to stop, continue, or start in order to improve coping strategies [15]. This comprehensive approach allows clients to take proactive steps towards recovery, fostering a more adaptive response to their traumatic experiences.

Broadening of Goals

In trauma therapy, helping clients expand their goals and set future objectives is vital for creating a pathway to recovery and long-term stability. This phase involves reflecting on the progress made during therapy, such as recognizing achievements and significant moments of personal growth, which can be particularly empowering for trauma survivors. Reflecting on their readiness to support others and outlining plans until the next session helps clients build a sense of agency and control, crucial for overcoming trauma [5].

Setting goals for the next 3–6 weeks, months, or years across the dimensions of body, work/achievement, social contact, and spirituality/futurity ensures a holistic approach to healing. This comprehensive planning addresses various aspects of a client's life, supporting their recovery from trauma by integrating their experiences into a broader framework of personal growth and well-being [13]. By focusing on these dimensions, clients can work towards a balanced life, where their trauma experiences are acknowledged and managed within the context of their overall goals and aspirations. The responsibility dimension found to be effective in motivating clients to be responsible in setting smart goals across four areas of life (Fig. 48.4).

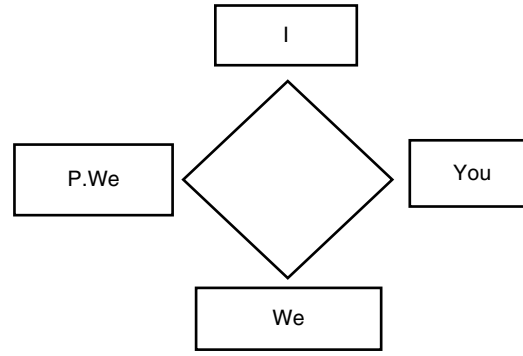


Fig. 48.4 Responsibility dimension

Clinical Vignette

The Ability to Develop Abnormal Response for Abnormal Situation

The client is a 39-year-old woman, working for a private organization. She is married and the mother of four children. Initially, she was admitted but showed minimal response to treatment. After 3 months of intensive medical treatment in the hospital, the client was referred to psychotherapy services.

In the distancing phase, the therapist followed the following procedures: Preparation, History taking, Establishing Therapeutic Alliance, Screening, Assessing the Window of Tolerance, Stabilization, and Identifying Actual Conflicts. During history taking, the client told the therapist that she was pregnant when a war broke out in her living village and she miscarried. She was unable to get skilled health attendants instead she got support from traditional healers. In addition, she lost her colleagues because of an air strike and severe war in the area. Furthermore, she wit-

nessed people who were severely burned, wounded, and killed in her village. She was repeatedly visualizing, remembering, and dreaming all terrific situations that happened in her village.

In the preparation, the therapist ensured safety for the client and checked if her environment is safe. Because of the client's strong mistrust capacity, establishing a therapeutic alliance was a challenge. Declaration of neutrality and confidentiality supported the therapist to get trust eventually.

Having established a therapeutic alliance, the therapist self-administered the Adverse Childhood Experience (ACE) questionnaire, and the modified PTSD symptom scale. In the ACE assessment she positively responded to seven questions and she scored 87 in the Modified PTSD symptom scale—where a score of 71 or greater is indicative of PTSD.

Then, the therapist assessed the window of tolerance and observed the following: hypoarousal symptoms including numbness, emptiness, blank stare, and inability to speak. After that, the therapist started to teach the client basic stabilization techniques in order to maintain an optimal level of window of tolerance. Accordingly, the therapist shared a client the following story from "The Little Book of Wisdom: A Collection of Fables and Proverbs" entitled "The Wise Old Man and the Broken Pot".

In a small village, there was a wise old man known for his deep wisdom and calm demeanor. One day, a traveler arrived at his home carrying two large pots, one of which was cracked and leaked water as it was carried. The traveler was distressed, lamenting the broken pot's inability to hold water.

The wise old man took the traveler on a walk along the path where the cracked pot was used. As they walked, the old man pointed out the vibrant flowers blooming along the path. He explained that the cracked pot had been watering these flowers, bringing beauty and life to the surroundings.

He said, "Sometimes, our flaws and challenges can lead to unexpected and beautiful outcomes. The pot's cracks allowed water to nourish these flowers, which would not have grown otherwise."

The traveler, realizing the pot's imperfection had created something lovely, felt comforted. The story illustrates that even our perceived flaws and difficulties can contribute to something positive and meaningful, helping to stabilize and reframe one's perspective on trauma and challenges.

Then, the therapist encouraged reflection about the story and the client responded that the story helped and soothed her, leading to a change in perspective. The therapist also taught diaphragmatic breathing exercises, part of Interval Training as developed by N. Peseschkian, and body scanning hypnotic relaxation by the author.

In Identifying Actual Conflicts the client is supported by the therapist in framing her actual conflict. The client felt that what happened to her and to the community and to the country in general were unacceptable for her. She wanted to see things continue as they were before. However the reality was different for her. Her internal needs and realities are fighting and creating conflictual tension. During the therapeutic process two principles were identified: Justice and Orderliness. She also escaped into isolation and totally avoided companionship.

In the inventory phase, the therapist supported the client making deep observations focusing on the following points: Understanding client's status in four major areas of life: Body, Contact, Achievement, Spirituality; Mapping out traumatic experience starting from childhood in order to get to a touchstone memory; understanding basic conflict, identify key conflict, and internal conflicts.

The therapist assessed the client's present status using the balance model approach. Accordingly, she feels disconnected from her body and emotions in order to cope with overwhelming stress. In addition, she was unable to express her emotions. She also had chronic pain in her neck area, back, and muscles. In addition, she was susceptible to repeated illness after the onset of this traumatic experience. She also had headaches, fatigue, and dizziness. She lacked motivation to work and also refrain from spiritual practice and she put her beliefs into question.

The therapist also helped the client map out her traumatic experience in order to get to a touchstone memory. The therapist used a mapping matrix to map her traumatic experience and the previous traumatic experiences already healed. Therefore, the process focused on working through the current traumatic episodes.

In order to understand the therapist considers the contents of the actual conflict to go deeper into understanding the basic conflict. The therapist was curious to understand if the client's excess demand of justice has a link with one of or the other basic capacities. During her childhood justice was a means to care for her parents especially for her father. At the same time, she suffered from injustice in the family during her childhood. Therefore, Justice is a compromised capacity for her. This capacity also affected her relationship seriously. Her Key conflict is Politeness.

The fusion of her actual conflict with the basic conflict awakens her internal conflict which should be solved at an early stage.

In the situational encouragement phase, the therapist encourages the client's strong capacities. Here the therapist brings validation pointing out that excess demand of justice is highly linked in protecting acceptance or love during childhood. It was a new insight for the client.

In the verbalization phase, the therapist focuses on her key conflict and verbalizes to his client's optimal level of honesty is crucial and also advised to work on her contacts. In the broadening of goals, clients are encouraged to create a plan that encompasses the four key areas of life: body, work, contact, and spirituality. In the body area, clients focus on physical well-being, such as improving health habits, fitness, and self-care routines. In the work area, the client plans to enhance career satisfaction, productivity, or skill development. In terms of contact, they look at strengthening relationships with family, friends, and social networks. Finally, in the area of spirituality, clients explore their personal beliefs, values, and purpose in life, setting goals that support their emotional and spiritual growth. By expanding goals in these dimensions, clients create a holistic plan for healing and growth that balances physical, emotional, social, and existential well-being.

Summary

The trauma healing process using Positive Psychotherapy (PPT after Peseschkian) is a well-designed approach that helped the therapist to develop different proven healing techniques. The therapy utilized a five-step consultancy approach namely distancing, inventory, situational encouragement, verbalization, and broadening of goals.

In the first step, distancing allows therapists to follow several technical procedures in the healing process. In order to create an organized and effective healing process the therapist expected to fulfill the following procedures: preparation, history collection, establishing therapeutic alliance, and screening, assessing window of tolerance, stabilization, and identifying actual conflict.

The second step, Inventory, comprises understanding client using balance model in four major areas of life: Body, Contact, Achievement, spirituality; map out traumatic experience of clients starting from childhood in order to get to touchstone trauma and helping client in order to understand basic conflict, identify Key Conflict, and become aware of Internal conflict.

In the third step, Situational Encouragement, clients are encouraged to recognize the positive coping strategies they already use in their daily lives. It further highlights four key elements mainly verification, resource mapping, using resources for self-regulation, and encouragement.

In the fourth step, Verbalization focuses on identifying what aspects of their experience are beyond their control and what they can change and this process supports clients in focusing their efforts on areas where they can make improvements and exert influence.

In the fifth step, in the Broadening of Goals, clients are expected to set new goals in connection with body, work, relationships, and spirituality. This aspect will help clients to regain a sense of purpose and direction in life.

Key Points

Priorities should be given to the following key points during the trauma healing process using practical techniques of Positive Psychotherapy (PPT after Peseschkian).

- Be informed that trauma is a normal response for abnormal situations.
- Make adequate preparation, appropriate history collection, establishing a therapeutic alliance, screening, assessing the window of tolerance, stabilization, and identify actual.
- Understanding clients using the balance model, traumatic experience matrix, basic conflict, Key Conflict, and Internal conflict is crucial.
- Measure disturbance level as deemed necessary.
- Trauma is stored in the form of pictures therefore use creativity to fasten the healing process.
- Understand core beliefs or thinking errors highly interwoven with traumatic experience.
- Identify emotions highly connected with traumatic events.
- Check body sensation in connection with emotion.
- Use verification, resource mapping, and using resources for self-regulation, and encouragement techniques.
- Use assertive communication for discussing change dynamics.
- Expand clients' goal across body, work, contact, and futurity dimension.

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The Challenge of Mental Health Professionals Being a Refugee and Working with Refugees: Tools and Approaches of Positive Psychotherapy to Work with Refugees

Liudmyla Moskalenko, Natalia Khanetska, and Larysa Hilova

Introduction

The issue of psychological and psychotherapeutic assistance to refugees is extremely relevant in the modern world with more than 100 million people worldwide having been forced to flee their homes because of torture, conflict, violence, and human rights violations. But there is a dearth of research exploring the factors promoting mental health resilience in asylum seekers and refugees [1]. In our opinion, being a refugee should be viewed not only as a state or experience of a person, but also as an objective social phenomenon—the social status of a refugee. The application of the principles of Positive Psychotherapy (PPT after Peseschkian) (the principle of hope, the principle of balance, the principle of consultation) and its main methodological components (positive approach, differential analysis, five-step model of therapy and self-help) makes it possible to carry out both a theoretical analysis of the psychological mechanisms of the formation of the social status of a refugee and to develop programs of counseling/psychotherapeutic support for this process.

Background: Refugee and Status

The term "status" is derived from the Latin verb "statuer" ("to place", "to establish") and is defined as an integral indicator of the position of a social group and its representatives in society, in the system of social hierarchy and social relations, which characterizes the system of rights, privileges and responsibilities that its bearer has [2]. The positive approach of Positive Psychotherapy ("positum") allows us to reveal the very essence of the concept of social status, considering it as a unity of "pros and cons", rights and obligations, opportunities and limitations of a person who occupies a certain place in the system of social stratification. According to

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_48

T. Shibutani, social status, no matter how low, is important because without it a person has no rights in relation to others. Possession of status allows a person to expect and demand a certain attitude from other people [3] and indicates the authority and prestige of the individual. Based on the theoretical model of the psychological conditions for the formation of a person's social status [4], the analysis of a certain social status involves the disclosure of its objective characteristics such as:

- The degree of authority and prestige of a status position, its place in the system of social stratification of a particular society at a particular time.
- The scope and content of rights, privileges and obligations of status holders.
- The degree of "assignment-creation", "formality-informality" of status-role functions.
- Specifics of social groups within which social status is emerged.
- Peculiarities of social status indication (clothes, housing, manners, ways of spending time of status holders, etc.).
- Evaluation of social status (demonstration of respect/disrespect, degree of politeness) and social sanctions in interpersonal interaction.

Particular attention should be paid to the stages of a person's transition from one social position to another, and to his or her new place in the stratification of society. The English ethnographer [5] described in detail the phenomenon of liminality, i.e., the stage in a person's life when the old position of a community member has already been abandoned and a new one has not yet emerged. One of the forms of manifestation of such liminality is the transition of a person to other social communities, in our case, to the status of a refugee or internally displaced person. From the point of view of PPT, such transitions, which cause a crisis of personal development, should be considered as macro-events on the person's life path, which can acquire the characteristics of macro-traumas.

Refugees and Immigrants

It is important to distinguish between refugee and immigrant status, because an emigrant is "from the point of view of the country of origin a person who has voluntarily left their country for economic, political, or personal reasons. This is a planned permanent departure in a state of psychological readiness to move to another country for permanent residence, characterized by a high level of understanding of the necessity for sociocultural and psychological adaptation. With a conscious choice of personal transformation" [6, p.16]. According to the *The 1951 Refugee Convention* [7], a refugee is a person who "owing to well-founded fear of being persecuted for reasons of race, religion, nationality, membership of a particular social group or political opinion, is outside the country of [their] nationality and is unable or, owing to such fear, is unwilling to avail [themself] of the protection of that country; or who, not having a nationality and being outside the country of [their] former habitual residence, is unable or, owing to such fear, is unwilling to return to it." As of 2024, according to official data of the United Nations (UN), there are more than 100 million people in the world who have been forcibly displaced due to armed conflict, violence, persecution, human rights violations or natural disasters [8]. Refugees are entitled to international protection under The 1951 Refugee Convention. Although European countries already have many years of experience in helping refugees, Russia's full-scale invasion of Ukraine in 2022 has caused a large new wave of refugees in Europe. According to official UN data, by the end of 2023, 3.7 million people remained displaced within Ukraine and 6.3 million Ukrainians remained abroad [8]. The 2024 Summer Olympics in Paris, along with teams from around the world, included a separate refugee team with its own flag and other indicators of their separate status, which for the first time (after the traditionally first team of Greece) was the second at the opening ceremony of the Olympics. Thus, the international community draws special attention to the social status of refugees in the modern world. Although the idea of asylum is generally supported in the world, the

arrival of a large number of refugees is a matter of concern for states. This has significant economic consequences: funds are spent on protection, food, clothing and housing for millions of people who have fled their homes. Rich countries that provide aid often have their own economic problems and express dissatisfaction that they cannot be responsible for all the world's homeless, especially when the problem is only getting worse. In addition, refugees face the phenomenon of xenophobia, as people in some countries believe that refugees threaten national culture, and this fear sometimes leads to violence. Therefore, the social status of a refugee usually does not entail authority and prestige. Although legally registered refugees have certain rights in the host country, in general, compared to the rights in their country of origin, these rights are significantly limited, even in terms of the possibility of free movement. At the same time, such legally registered refugees have a number of obligations, including language learning and employment, the fulfillment of which is a condition for granting asylum. The social status of a refugee is identified by the place of residence (e.g., a shelter), the use of their national language and lack of proficiency in the official language of the host country, external features that distinguish representatives of a particular nationality, and many others that immediately identify a person as a refugee. If the main task of a person in the status of an emigrant is to adapt to living in a new country, a refugee faces two tasks simultaneously: in addition to adaptation required "externally" by representatives of the host country, a person needs to grieve and process the life losses he or she has suffered, and to heal the traumas received in the process of refugeeism, as their psyche requires it "internally".

Forced displacement for a refugee becomes a macro-trauma in every sphere of life due to living with multiple losses that last over time: deprivation of living standards, lifestyle, professional role, loss of necessary external resources, status, lack of economic support, etc. In addition, the social status of a refugee leads to a fundamental change in the form of micro-traumas in each of the spheres of life, starting with changes in the daily routine of caring for one's body (the Body/Health sphere). Forced migration in times of war can be viewed as a liminal situation according to K. Jaspers—a situation of despair, guilt, horror that is born at the moment of meeting death, when a person is threatened with moral or physical death. According to K. Jaspers, it is through the shocks of the ultimate situation that a person finds freedom and transcends the everyday "object being". The new social status of a refugee is characterized by a certain amount of new rights and obligations in each sphere of life. These rules are designed to facilitate integration, familiarize with the norms and values of the host country and ensure that people in need of protection comply with legal requirements. Integration covers various areas, from language support and participation in cultural life to access to the labor market and active social/transcultural interaction. The number of rights and obligations of a refugee can be manifested both in a balanced way, when a person spends energy more or less proportionally in all areas of life, and in an imbalance, when some areas are significantly overloaded and others underloaded. Mastering the language in the host country and the associated employment opportunities become paramount tasks.

Refugee Adaptation and the PPT Balance Model

The following main areas of refugee adaptation in the host country can be identified on the basis of the life spheres model (See Chap. 8):

1. Physical adaptation (climate conditions and time zone change, food, new sensory stimuli: sounds, smells, etc.)
2. Organizational adaptation (legal documents, household arrangements, access to finance, employment/education, etc.)
3. Socio-cultural adaptation (language, cultural norms and customs, traditions and rituals, holidays, etc.)
4. Worldview adaptation (values, beliefs, worldview)

A 44-year-old psychotherapist from western Ukraine, who spent 2 months living in a refugee shelter in Poland following the 2022 Russian invasion, described her adaptation experience across several areas:

Body/Health
She faced significant challenges adjusting to shared living conditions, including the lack of private space, limited access to basic hygiene facilities, and restricted opportunities for personal grooming. Refugees had to make do with minimal clothing and personal care items, which affected their sense of identity and self-expression.

Achievement/Work
Adapting to the shelter's daily routine required coordinating tasks with others and adjusting work habits. For instance, remote work was difficult due to the lack of quiet, private spaces. Scheduled mealtimes and communal living left little flexibility for personal or professional needs.

Contact/Relationships
Language barriers and cultural differences often made communication difficult and sometimes strained relationships among residents. Even refugees from the same country had varied backgrounds, leading to misunderstandings and discomfort. Conversations frequently revolved around shared traumatic experiences, and privacy was almost nonexistent.

Meaning/Future/Values
She observed differences in how residents understood and coped with the war, influenced by their backgrounds and personal losses. Those who had experienced more direct violence or loss often had different needs and perspectives than those who left earlier or under less severe circumstances.

Unlike an emigrant who consciously chooses a new country, studies it in advance and prepares for adaptation in all areas, a refugee in most cases has no choice of a country of asylum, and the move is quick and unprepared.

Describing the main actual conflicts that arise in a refugee's life as a result of macro and micro events, we can identify the following. Thus, in the area of the "Body/Health", there is often a certain physical incapacity, loss of care, loss of opportunities to meet basic needs (food, sleep, routine, etc.). Refugeeism as an extreme experience exacerbates the feeling of not having a place: home, country, living space, habitual way of life, sensory stimuli—thousands of small things and objects that saturated the aesthetics of everyday life with beauty and love.

In the "Work/Achievement" area there is a loss of work, educational and professional status and income, lack of career growth or advanced training, difficulties with learning a new language and fear of language exams, the need to perform low-skilled work, etc.

In the area of "Contact/Relationships", there are frequent divorces, conflicts in partnerships and families, lack of support in raising children, and various barriers to communication (political, religious, and linguistic), inability to contact nature, animals, etc. The loss of meaningful relationships (separated families or loss of friends) revives basic conflicts and retraumatizes experience of isolation or rejection from the childhood.

In the area of "Meaning/Future/Values," we observe a sense of losing one's "roots," the inability to return to the place of birth and other subjectively significant places, loss of memories (loss of photos, videos, and memorabilia from previous life), disruption of plans, fear of the future, lack (loss) of planned opportunities, loss of choices, etc.

Although acquiring refugee status can lead to a number of actual conflicts in each of the four areas of life, such actual conflicts do not necessarily lead to actual inner subconscious conflicts. On the contrary, some actual inner subconscious conflicts can be resolved by changing the actual situation (See Chap. 35). *For example, on the one hand, the separation of a refugee wife and her husband who stayed in the homeland can lead to the emergence of an actual conflict in the "Contacts/Relationship" domain because of the actual capability "fidelity". On the other hand, a forced move to another country can be a solution*

to an actual conflict in relations with the mother-in-law regarding the actual capabilities "obedience", "cleanliness" and "orderliness": living on her own, a woman does not gain daily microtraumas. This "resolution" of actual conflicts leads to a decrease in the tension of actual inner subconscious conflict and a decrease in symptoms, which we observe in some refugee patients/clients. (**From the experience of working with Ukrainian refugees in Europe as a result of Russia's large-scale military invasion in 2022.)

Difficulties with refugees' adaptation to living conditions in the host country, on the one hand, and the internalization of traumatic experiences and multiple losses, on the other, lead to a number of symptoms. Symptoms are exacerbated if actual conflicts activate the basic conflict of the person. In this case, symptoms should be seen as unconscious ways of coping with the situation. Common symptoms among refugees include: changes in eating behavior, fatigue, alcohol and substance use, weakness, illness, lack of normal sleep and sex patterns, cognitive impairment ("Body/Health" area), workaholism, lack of desire for activity, lack of energy, procrastination, low motivation, rebellion, sabotage, fatigue from a specific activity, fatigue from the number of activities, wastefulness or savings, constant reading of the news ("Work/Achievement" area), quarrels, extreme manifestations of introversion/extraversion, solitude, "freezing" of emotions ("Contact/Relationships"), fears, failure to plan for the future, loss of meaning, unconscious automatic living of days, escape to watching movies and videos, indifference ("Meaning/Future/Values" area)*.

In our work with refugee patients/clients, we use the metaphor of a house: just as war can destroy the walls of a house to the very foundation, a person's previous life balance can be completely destroyed. On the one hand, the inability to satisfy important needs, such as professional fulfillment in a new country, is a common actual conflict for refugees, on the other hand, the forced way of life as a refugee in another country can teach a person a different way of redistributing life energy and change their concepts, for example, to learn to pay more time and attention to the needs of the body ("Body/Health" area) and communication with loved ones ("Contact/Relationships") as opposed to workaholism in the previous lifestyle ("Work/Achievement").

Thus, we can also describe the resources (events, changes, manifestations) observed during the formation of a new refugee status: bodily stabilization, physical activity, traveling, walking ("Body/Health"), active work, new activities, interesting studies, new investments, charity, volunteering ("Achievement/Work"), social support, breaking off old unacceptable relationships, reconnecting with childhood friends, consultation and helping others, socializing in new groups, contact with nature and animals, observing national traditions, introducing others to your culture and traditions ("Contact/Relationships"), faith, religion, self-exploration, manifestation of one's own self in different (new) identities (realization of old dreams), research of the past (own, country, family), feeling part of the national community, development and preservation of national identity, expansion of the diaspora, search for the meaning of death (heroism), research of existential data and expansion of existential possibilities, prioritization of values, search for a situational and universal mission, consumption and study of art, creativity, etc. (Meaning/Future/Values)*.

Differential analysis of refugee status based on the conflict model in Positive Psychotherapy (as *subsection title*) Looking deeper into the psychodynamic processes of becoming a refugee, we can say that due to the loss of previous social selves, there is a certain degree of rebirth of the personality with opportunities for its development and risks of losing physical and mental health (and even life).

Positive Psychotherapy identifies three main factors in personality formation: bodily factors, the influence of the social environment, and time (See Chap. 2). In the case of refugee status, a person (1) is in a state of acute stress and physical exhaustion, often having injuries, etc. (2) finds himself/herself in a completely different social environment in the position of a dependent (3) without any time certainty about their future (when the war or persecution will end and they can return, whether their residence permit will be

extended, etc.). The complete physical, financial, and legal dependence of a refugee returns him/her to a childlike position in his/her relationships with others in the host country, to the stage of "attachment" (See Chap. 2), where one of the main conditions for satisfying emotional needs for love (primary actual capabilities of love, trust, patience, time, etc.) is such socially approved behavior patterns as obedience, diligence, politeness, etc. (secondary actual capacities as values). It is this forced return to dependence on others that reactivates basic personality conflicts and previous traumatic experiences.

For example, a client with a basic conflict in the sphere of "I" contact-obedience experiences severe anxiety in communication with a social worker. Due to her insufficient knowledge of a foreign language, she is afraid that she will not understand everything and will not do "the right thing, the way she was told" and that the social worker will be dissatisfied with her, will not want to help her, or will even abandon her. This anxiety is not caused by the actual actions of the social worker, but by the client's childhood experience when her parents demanded obedience from her and threatened to abandon her in case of disobedience and send her to an orphanage.

A client with a basic conflict in the sphere of "I" love-justice always feels that other refugees with whom she arrived on the same bus are treated better by the locals: they live in families with better living conditions, their children are enrolled in a better school, etc. Her reactions are the result of her childhood experience: having an older brother and a younger sister, she always felt that they got better treatment from their parents: her sister because she was small, and her brother because he was older and in charge. By comparing herself to the refugees from the same bus, she unconsciously gets into the experience of a basic conflict about justice."

A refugee is forced to learn a completely new way of life (the basic capacity to "know") and to build his/her system of social interaction in a new way (the basic capacity to "love"). N. Peseschkian emphasizes the importance of balancing the components of these capacities: the capacity to learn and the capacity to teach, the capacity to give love and receive love [9]. The peculiarity of the refugee's social position is that in adulthood, despite all their previous life experience, a person finds themselves in the position of someone who has to learn and take (even in basic matters of food), but has few opportunities to teach and give. This imbalance creates inner tension. *Our experience of working with refugees* shows that opportunities to teach and give love to other people in the host country (volunteering to work with their fellow citizens, engaging in cultural exchange activities,* etc.*) have a positive impact on the mental state of clients and help reduce symptoms.* At the same time, the forced experience of returning to the stage of attachment in the transcultural four model dimension of the host country gives the individual a chance to change basic family concepts of relevant secondary capabilities, to make them more flexible and differentiated. The leading role in this process is played by transcultural experience (See Chap. 21). *For example, we observe cases when a client with a basic conflict "love-achievement" in the "I" dimension and who does not accept herself as unsuccessful (with a low social status, financial situation and professional career), demands a high level of academic achievement from her children, and ends up in European countries with a different value system, such as the Scandinavian countries, finds that such a secondary capability (social value) as achievement is not a prerequisite for building relationships: the people around her give her acceptance, regardless of educational, professional, or financial status.*

In our opinion, it is the transcultural approach of Positive Psychotherapy based on the principle of hope that makes it possible to discover the significant psychodynamic potential of the refugee's social status. Due to the childhood experience of one's culture (family, region, nationality, etc.), a person has formed certain differentiations of basic capacities and actual capabilities that have acquired signs of discrimination. N. Peseschkian writes that significant adults in childhood play the role of switchboards that direct the basic capacities to know and love in some directions and restrain them in others [9]. Fixed in basic concepts, such discriminations become unconscious

factors of human behavior in adulthood. The transcultural approach answers the question: how are all people similar and how are they different? Emotional needs (primary actual capabilities) are common to all people, but the socially approved ways of satisfying them (secondary actual capabilities), learned in their culture, are different.

For example, a mother's "Don't come to me, you're dirty" fixes in the child's mind the connection between primary (emotional needs) and secondary (social norms) capabilities—"contact—cleanliness": if you want others to contact you, you must keep yourself clean; dirty people are not worthy of contact. In this case, the actual capability of "cleanliness" limits the manifestation of the primary actual capabilities and the basic capacity to love in building relationships with oneself and others, and the fear of getting dirty and not being worthy of love limits the manifestation of the basic capacity to know through sensations. In adulthood, such basic concepts lead to discrimination in relationships with others, interpersonal conflicts, and internal conflicts if the desired body cleanliness cannot be achieved or an attractive object is dirty. The transcultural experience of living in another country makes it possible to differentiate strong unconscious connections between primary and secondary actual capabilities: love exists separately and cleanliness exists separately, it is possible to hug someone who is dirty (the experience of raising children in Scandinavian countries, when children play freely in puddles) and it is possible to have a clean body that does not guarantee warm emotional relationships with other people.

Given that the refugee is in a dependent position on the host country, we assume that the new social environment can act as a "parent" in the Model Dimensions (See Chap. 2), giving the refugee a new experience in all dimensions: a new experience of the "I" attitude, building partnerships "You", relationships in groups "We" and the "Primordial We" attitude to the world, weakening the limitations of the basic concepts of their culture. Therefore, according to the humanistic approach, it is necessary to consider the individual as a subject of social status who can not only adapt to new living conditions but also use them for growth and development. From the perspective of counseling and psychotherapeutic practice, we are interested in how the social status of a refugee can affect the psychological and mental health of a particular person, and how psychotherapeutic support and assistance can be provided to prevent and overcome negative consequences. The way a particular person will live the refugee status is determined by the interaction of external and internal conditions. It is the differential analysis of the refugee's social status in terms of Positive Psychotherapy that makes it possible to identify conflict and resource areas using the concepts of spheres of life, basic capacities and actual capabilities, and personality concepts.

According to the theoretical model of the formation of a new social status of a person [4], external objective conditions for the formation of a refugee's social status are determined by the environment: 1) the material conditions that determine the exercise of the refugee's rights and obligations (according to the balance model), and 2) his/her reference circle of communication (according to the relationship model), which determines the completeness, adequacy and consistency of the description of expected behavior, requirements for it, its assessment and sanctions. External material conditions can be as follows:

- "Body/Health": physical conditions of stay in the host country, preservation or loss of one's own material resources (housing, savings, valuables, personal clothing, favorite things, etc.), the ability to meet basic needs for food, water, sleep, etc., access to medical care, etc.
- "Achievement/Work": official status in the host country, financial assistance, recognition of education and profession, opportunities for education and employment, etc.
- "Contact/Relationships": presence of family members and their living conditions, social environment in the host country, language proficiency, ability to maintain contacts with relatives and friends in the home country, etc.
- "Meaning/Future/Values": prospects for ending unfavorable conditions in the home coun-

try and opportunities for return, availability of assistance programs for refugees in the host country, providing stable step-by-step planning and a vision of a possible secure future.

In addition to material conditions, the formation of a refugee's social status will be influenced by reference persons who set the framework for a person's interpretation of his or her refugee experience:

- The sphere of "I": whether a person is a reference person for himself/herself, how much he/she trusts his/her decisions and has a stable position regarding them.
- The "You" sphere: what is the attitude of the client's partner towards forced displacement and the host country.
- "We": what is the attitude of the family (parents, siblings, children of the client), friends, colleagues, fellow villagers, community members, etc. toward forced displacement and the host country; what is the attitude of the new social environment in the host country, etc.
- "Primordial We" sphere: what attitude to the refugee situation and the host country is reflected in the client's religion, worldview, and transgenerational experience.

The internal conditions that determine the formation of a person's social status are the degree of formation and coherence of the social position and social identity of the individual. Social identity is seen as an individual's awareness, feeling, and experience of belonging to a particular social community. By social position, we mean a system of knowledge and attitudes of a person to his or her status [4]. Taking into account the unity of two basic human capacities in interaction with the world: to know (cognitive) and to love (emotionality) according to N. Peseschkian, considering social position and social identity of a person as a complex formation, we define cognitive and emotional components in their structure.

The refugee's social position includes a cognitive component (a system of knowledge and ideas of a person about the totality of his/her status rights, privileges, moral norms and responsibilities, the degree of prestige and authority of his/her position) and emotional components (assessment of his/her social position as a refugee and attitude towards it). Social identity is also a unity of two components: cognitive (awareness of belonging to the refugee community) and emotional (place of refugee status in the hierarchy of social roles of a person, assessment of one's belonging to the refugee community).

As for the cognitive component of the refugee's social position, it is extremely important to have an adequate, complete and consistent system of knowledge about one's status. An important component of this knowledge is a person's understanding of the social norms (secondary actual capabilities) that are expected of him/her as a refugee and as a resident of a new country in general. Knowledge about such social requirements, expectations and sanctions can be gained through four channels of cognition: senses, reason, traditions and imagination (See Chap. 2): what sensory (positive and negative) experiences a person has had from being in another country; what information he or she has about the country and his or her status in it, his or her rights and responsibilities, opportunities and limitations; what traditions say about refugee status and the new country; what are the person's fantasies about refugee status, the new country and his or her future in it. A Ukrainian psychotherapist who spent time in a Polish refugee shelter described how the physical toll of displacement became evident after several weeks. She experienced persistent physical discomfort—such as stomach issues, dry skin, back pain, and fatigue—which made her realize the importance of reestablishing routines that supported her well-being. *She experienced persistent physical discomfort—such as stomach issues, dry skin, back pain, and fatigue—which made her realize the importance of reestablishing routines that supported her well-being. She began making adjustments by seeking out familiar foods, finding pleasant places to walk, and using local body care products and medications, eventually regaining a sense of physical comfort.*

Financial limitations also made her reflect on her professional identity. The lack of income prompted her to explore how her qualifications and therapeutic work could be adapted to local

legal and cultural frameworks, including understanding regulations, client support systems, and professional responsibilities in Poland.

She also shared her experience of navigating cultural differences in communication. A simple interaction with a Polish doctor highlighted how speech patterns could be misinterpreted—what initially seemed like uncertainty or informality was later understood as a common linguistic feature in Polish, not an actual request for consent or guidance.

Finally, her initial belief that assistance for Ukrainian refugees relied solely on individual generosity shifted after observing the extensive, well-organized governmental and institutional support systems in place. This realization helped her better understand the structure and scale of Poland's commitment to supporting refugees.

The emotional component of a refugee's social position can be described through the primary actual capabilities of the individual: acceptance or rejection of refugee status, living conditions and the new country in general; hope or hopelessness about one's future as a refugee; trust or distrust in the new social environment and social institutions, time (time uncertainty), and others. These attitudes are determined, on the one hand, by the adequacy and completeness of information about their situation and country of residence, and, on the other hand, by previous life experiences reflected in the concepts of the personality in the areas of "I", "You", "We" and "Primordial We". A Ukrainian psychotherapist described the early weeks in the shelter as being marked by a deep sense of temporal uncertainty, which she found to be one of the most difficult aspects of displacement. While she was grateful for the support and held onto hope and trust—in society, in the armed forces, in faith, and in herself—she struggled with not knowing how long it would take to emotionally adjust.

Her past beliefs, shaped by stability and agency, had centered around the idea that she could influence her circumstances and that personal effort would lead to desired outcomes. She also held strong interpersonal values—believing, for example, that partners should remain together and that people should treat one another with mutual respect and dignity. Spiritually, she was guided by a sense that divine protection would prevent the worst from happening.

However, these foundational ideas no longer aligned with her current reality. Despite being able to assess her situation rationally, she found herself yearning for the certainty and coherence of her past worldview. The ambiguity of time—how long this new, unsettling phase would last—made the adjustment especially hard. Ultimately, what brought her clarity and relief was working with her own therapist to expand and adapt these core beliefs to better fit her lived experience in displacement.

The more adequate, complete, and consistent a person's knowledge of his/her social position in the host country and life in this country is, the more adequate the person's attitude to his/her situation and the host country in general is, the easier the process of adaptation is. As for the cognitive component of social identity, its important characteristic is certainty, i.e. a clear understanding of a person as a holder of the social status of a refugee. It is the certainty of social identity that promotes meaningful, positive interaction with the social environment [4]. *We observe many cases* where refugees avoid calling themselves refugees, trying to find other social statuses for themselves. For example, clients call themselves simply guests in the host country or envoys of Ukraine in Europe, when in fact they are refugees. Another part of the clients avoid identifying with the status denoting the right to reside altogether, calling themselves neither refugees nor migrants: by continuing to work online at their place of work, receiving a salary and having contact mainly with Ukrainians, they create for themselves a certain illusion of their social status.*

Applying the relationship model in Positive Psychotherapy, we can talk about the certainty or uncertainty of a refugee's social identity in four areas:

- The sphere of "I": Who am I when I am alone with myself?
- The sphere of "You": Who am I to my partner and in my relationship with my partner?
- The "We" sphere: Who am I in this country? Who am I in my country? What do my people like? Who am I in my communities (in the

diaspora, in a language learning group, in a workplace)?
– The sphere of "Primordial We": Who are my ancestors? What did they live for? Who am I in the world and before God (the Universe, the planet)? Who am I in the future?

Physical presence in a group does not automatically lead to a corresponding identity. The main mechanism of social identity is the comparison (or opposition) of interests, views, values, assessments, behavioral patterns of one's group (community) with those that are considered not one's own (or hostile), whose interests are indifferent to this community or conflict. Here we can observe that for the citizens of the host country, all refugees from the same country are perceived as one community, but for the refugees themselves, other people from their country are not automatically close. *In our practice of working with refugees*, we often observe cases when the population of small towns and villages wants all Ukrainians living there to communicate with each other as one community. For Ukrainians themselves, other people from Ukraine can be even more alien than foreigners, because such closeness is determined not by nationality itself, but by the coincidence or mismatch of values. Clients often say that, on the contrary, they would like to stay away from certain people from their own country and perceive foreigners' intentions to make them one community as violence against them.* The emotional component of a refugee's social identity characterizes the emotional assessment of one's identity and its place in the system of social roles of a person. Due to the polarization that occurs between those representatives of the country who went abroad and those who stayed at home, depending on the concepts of personality, the social identity "I am a refugee" can acquire both negative connotations "I am a fugitive," "I am a coward," "I am a traitor," and positive connotations "I am a good mother because I care about the future of my children, they live in a peaceful country," etc. Similarly, the social identity associated with the host country can acquire positive or negative connotations. For one client, "I am German" means that I am a successful person, while for another, "I am German" provokes a strong internal conflict due to the tragic experience of her parents' family during World War II. A change in the social status of a person (a clash of new and established knowledge about oneself) leads to an identity crisis (according to E. Erikson) if the old model of identity does not allow a person to integrate into the system of social relations and satisfy his or her basic needs. The more adequate and holistic the cognitive and emotional components of a refugee's social identity are, the easier the adaptation process is.

Working with Refugees Using PPT's Five-Stage Model

The five-stage model of therapy and self-help in Positive Psychotherapy can be used to build a strategy for psychological support and psychotherapeutic assistance to a person who acquires refugee status. As mentioned above, acquiring refugee status is a macro-event in one's life, and the stress of adapting to new living conditions can lead to certain symptoms at the symptomatic level, but it does not always mean the emergence of an actual inner subconscious conflict and the need for psychotherapy.

We can talk about three levels of support for refugees depending on their needs:

1. Work at the cognitive-behavioral level. Social and psychological support for adaptation. Coaching.
2. Work at the level of conflicts (actual conflict, partially inner conflict, key conflict). Psychological counseling.
3. Work on the structural level. Psychotherapy. Deep work with traumas acquired in the process of refugee and retraumatized previous experiences. Work with the personality structure and basic conflicts. Work with inner conflicts that have signs of neuroticism and interfere with a person's adaptation to a new social status [10].

Each of the five steps of the five-stage model of therapy and self-help according to N. Peseschkian (See Chap. 2) can be applied at the three levels of refugee support work, but with different depths of content.

Observation/distancing allows one to look at the refugee's social status from different perspectives, to form a more holistic and comprehensive understanding of it. The inventory is aimed at finding and analyzing internal obstacles to a person's adaptation to the social status of a refugee. Situational encouragement is aimed at identifying the person's resources in living and becoming a refugee, finding and realizing the positive aspects of the new social situation and its opportunities. Verbalization is focused on finding directions and ways to resolve current and internal conflicts caused by refugee status. Broadening of goals is aimed at setting new goals by the individual [2]. Let's take a closer look at the possible content of work at each stage. Observation and Distancing In order to form a holistic and versatile perception of social status, N. Peseschkian's methods of Positive Psychotherapy such as transcultural approach, positive interpretation, parables, and metaphors are used. The transcultural approach allows us to consider the social status of a refugee from different perspectives, in particular, historical, cultural and ethnocultural. The historical and cultural aspect includes the following questions: "What unites all refugees from the past to the present?", "How have refugees at all times differ from the indigenous population?", "How did people treat the social status of a refugee at different times?", "What opportunities did refugees have and what difficulties did they face?", "What is the peculiarity of refugees today?". The ethno-cultural aspect is revealed in the following questions: "How do different countries treat the social status of a refugee?", "What opportunities do refugees have in different countries? What difficulties do they face? How do they overcome them? "What is the attitude to refugee social status in this country?", "What opportunities do refugees have in this country? What difficulties do they face? How do they overcome them?"

Positive interpretation is carried out according to the levels described by V.I. Karikash [11]. The "Optimism" level is aimed at a situational change in the emotional state of a person in order to awaken "vital emotional tone" ("What are the advantages and opportunities of refugee status? Where, when, how does a refugee feel good?"). The "Creativity" level is aimed at developing the ability to make efforts to find a deeper, hidden meaning and significance of an obvious problem, the ability to identify "The other side of the coin" ("What does refugee status give and take away?"). Level "Realism" is aimed at developing the ability to distinguish the reality of the past, present and future from the illusions of the past, present and future ("Where is your reality in understanding refugee status, and where is the reality of other people? What is real in your perception of it and what is illusory? What do you have in common with other refugees and how do you differ from them?"). The "Integrity" level aims to develop the ability to combine disparate parts into a whole, to "maintain and restore balance" while remaining oneself ("In what ways does a person remain himself/herself in the new refugee status? How can they feel their integrity and remain true to themselves?"). The "Spirituality" level is aimed at developing the ability to acquire goals, values and meanings of a higher order ("What is the human purpose in the world? How can it be realized through the social status of a refugee?"). In the process of working with the client, metaphors reflecting the social status of the refugee, proverbs, aphorisms, parables that the client knows from his/her own culture, those inherent in the culture of the host country, those known to the counselor/psychotherapist from transcultural experience, etc. are also analyzed.

Inventory Stage

The "Balance Model", "Differential Analytical Inventory" (DAI), "Model Dimensions" and others are used at the inventory stage. A number of questions can be sequentially processed both regarding the social status of a refugee in general and a refugee from a particular country to a par-

ticular country under certain circumstances (for example, a Ukrainian refugee in Germany as a result of Russia's military aggression):

1. How is the social status of a refugee reflected and manifested in four areas of a person's life: "Body/Health", "Work/Achievements", "Contact/Relationships", "Meaning/Future/Values"? What are the refugee's advantages, disadvantages, opportunities, rights and "cons", restrictions, responsibilities?
2. What are the main macro- and micro-traumas that forced the refugee to change their country of residence? What macro- and micro-traumas are being experienced now?
3. How is energy distributed in the four spheres of life? What was the previous balance at home? What opportunities does transcultural experience provide for changing the life balance?
4. What ways does the person process conflicts according to the areas of the balance model? What methods did he/she use before? What are the possibilities of conflict processing offered by the current transcultural experience?
5. What secondary actual capabilities is the refugee social status associated with the activation and development of: punctuality, orderliness, cleanliness, thrift, justice, obedience, honesty, politeness, fidelity, reliability, accuracy, conscientiousness, diligence? In general and in a particular host country? What is the level of their development in a person? What previous experience does the person have with them? What basic concepts reflect this? What primary actual capabilities are expected of refugees as a reaction to their own social situation in the new host country and in their homeland (patience, time, social interaction, role model, trust, hope, sexuality, love, confidence, faith, doubt, unity)? What is the level of their development? What previous experience does the person have with them? What basic concepts are reflected in them?
6. What ways of knowing (basic capacity to know) does a person use to learn about a new country and understand their status? What cognitive experiences did he/she have in the past? What are the possibilities of developing his/her ways of knowing offered by the current transcultural experience?
7. What are the main goals, values and meanings for a refugee in four dimensions? "I" (own life and attitude to oneself), "You" (interaction with a partner), "We" (interaction with the environment), "Primordial We" (interaction with society and the world in a broad sense)?
8. What is the balance of learning and teaching, giving and receiving love in refugee status? What experiences have you had in the past? What opportunities does transcultural experience offer?
9. In what everyday situations can a refugee feel in the position of "I-" and "I+"? What areas of life and actual capabilities do they relate to the most? What previous experiences are they associated with? What basic concepts do they reinforce?

Situation Encouragement

The third stage—situational encouragement—focuses on identifying the individual's resources in living and establishing a refugee social status, finding and realizing the positive aspects of the new social situation and its opportunities.

1. What gives you hope?
2. What everyday little things bring you joy?
3. What new and valuable things did you discover in yourself while living the refugee experience? In your partner? In other people? In the world?
4. What can you be grateful for? Your partner? Family members? Friends? Your neighborhood? Other people? To the world?
5. What meanings have you already found for yourself in your situation? What meanings can still be found?

Verbalization

The fourth stage—verbalization—is aimed at finding directions and ways to resolve current and internal conflicts of the individual caused by refugee status.

1. What capacities, capabilities, knowledge, and skills do you want to develop in the area of well-being, health, appearance, activities, contacts, meanings, and future prospects?
2. What transcultural experience do you want and can use for your life?
3. With whom and what should you talk openly? What do you want and need to say?
4. What experience are you ready to work on further in the process of psychotherapy?
5. How will you feel? What will be the results? What will it give you? How will you do it? Who will you cooperate with? What or whom will you ask for help? What are you willing to overcome? What are you willing to spend? What are you willing to sacrifice? How will you know if your plan is working? How will you reward yourself? When are you ready to start?

Broadening of Goals

The fifth stage—broadening the goal system—is focused on setting new goals by the refugee:

1. What kind of help and support will you be able to provide to other refugees in the future? What useful experience can you share with future generations?
2. What long-held dreams and desires could you realize in a new country?
3. What new values and mottos would you like to implement in each of your areas of life? What plans do you have for the near future?

Summary

Modern science and practice calls for a comprehensive interdisciplinary and transcultural research on the needs of refugees and for effective psychotherapeutic methods of providing it. Positive psychotherapy (PPT after Peseschkian), as a meta-approach in psychotherapy, can become the foundation of such programs.

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49 Psychotherapy During War: Working with Soldiers and Separated Families

Liudmyla Moskalenko, Natalia Khanetska, Oleksandra Nizdran, Olena Lutsenko, and Svitlana Pidlypska

In modern society, military conflicts are becoming increasingly prevalent. Issues such as the nature of war, the psychological challenges faced by veterans, and the impact of combat are drawing growing attention from the psychotherapeutic community, highlighting mental health concerns [1–5]. Not only professional military personnel but also civilians involved in these processes require focused observation and study of psychological phenomena arising from "militarized violence." On one hand, many countries with professional armies have programs for rotation-based recovery and post-service reintegration, which have proven effective for professional soldiers. On the other hand, the involvement of large numbers of mobilized individuals in military operations and their subsequent return home—with physical injuries, amputations, PTSD, and other symptoms—presents a significant societal challenge. In the late twentieth century, the Vietnam War and the subsequent reintegration of American soldiers spurred advancements in psychotherapy, particularly group therapy. Today, Ukraine faces the critical task of supporting millions of both professional and non-professional military personnel and their families due to Russia's invasion. Positive Psychotherapy (PPT after N. Peseschkian), with its holistic approach to human beings and psychotherapeutic care [6, 7], provides a comprehensive framework for addressing the needs of military personnel and their families. This chapter shares the experiences of Ukrainian positive psychotherapists working with military families from 2014 to 2024.

From the perspective of PPT on conflict dynamics (actual, internal, basic, and key personality conflicts, see Chap. 35), war represents an array of traumatic macro- and micro-events that deeply affect the human psyche. These include massive shelling, physical and sexual violence, loss of loved ones, injuries, destruction of homes and property, displacement, occupation, and captivity. The frequency, intensity, and proximity of

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_49

these events to the individual determine the level of psychological conflict and stress. A distinct category of individuals whose lives have been profoundly affected by war includes military personnel, veterans—especially non-professionals such as mobilized individuals and volunteers—and their families.

Military personnel (combatants) experience significant physical and psychological stress while serving under martial law, particularly on the front lines, where they face combat stress and extreme survival conditions. War, as an extreme event, fundamentally disrupts people's lives, and the PPT balance model [7] can assist in mapping these disruptions. These stresses and experiences can be categorized into areas of the balance model, allowing for an assessment of their impact on an individual's life. For instance:

- **Body/Health**: Limited sleep and nutrition, prolonged exposure to harsh weather in trenches, sensory stress from noise, shock, chemicals, confined spaces, injuries, and pain.
- **Achievement/Work**: Loss of previous civilian jobs and income, the need for rapid adaptation to military roles, and related challenges.
- **Relationships**: Separation from family and loved ones, fear for their safety, constant proximity to fellow servicemen, inability to be alone, interpersonal tensions, conflicts, and grief over the loss of comrades.
- **Meaning/Future/Values**: Abandonment of pre-war plans and visions of the future, the constant threat to life, and confrontation with death.

Additionally, some military personnel face traumatic experiences such as captivity, torture, and physical or sexual violence. Many sustain severe injuries that result in lifestyle changes, disabilities (e.g., amputations, loss of vision), or altered appearance (e.g., burns).

From the perspective of Positive Psychotherapy as a psychodynamic approach, exposure to actual conflicts does not inherently result in mental disorders. The onset of symptoms and disorders is linked to the emergence of inner conflicts, arising from the clash of current experiences with past traumas (often childhood or earlier traumatic events). The unconscious nature of these inner conflicts can lead to feelings of hopelessness, reflecting the psyche's attempt to cope with the resulting symptoms. Thus, while combat experiences exert a profound influence on the psyche, they do not inevitably cause mental disorders. Instead, they amplify pre-existing unconscious contradictions within the personality.

Common psychological consequences of participating in hostilities, as widely documented, primarily include depression, anxiety, and alcohol abuse, with PTSD being less prevalent [1]. Characteristic post-stress conditions in military personnel include suicidal thoughts, which may lead to suicide; a persistent need to carry weapons and use them to resolve conflicts; loss of life's meaning and confidence; feelings of depression, pessimism, abandonment, and uselessness; distrust of others and difficulty speaking openly about wartime experiences; a sense of unreality regarding war events; feelings of helplessness to influence events; inability to be sincere and open in communication; excessive anxiety; a subconscious perception of having "died" in the war; guilt for surviving when comrades did not; anger and aggression about combat experiences; a constant need for adrenaline and excitement; and rejection of veterans of other wars. Psychosomatic disorders such as sleep disturbances and chronic muscle tension are also common [2].

Mental health challenges are widespread not only among military personnel and veterans but also within their families. These issues stem from ongoing conflicts that affect the family both during military service and after the servicemember's return. Long separations, anxiety for a loved one's safety, and difficulties in communication after their return often lead to family conflicts (the "Relationships" area).

Veterans also face challenges adapting to civilian life, including a shift in their social role (the "Meaning/Future/Values" area), lack of support, and employment issues (the "Work/Achievement" area). While the servicemember is

away, their spouse often assumes the role of head of the family, which brings increased responsibility, physical and psychological strain, and financial difficulties. Upon the servicemember's return, their psychological state and values often do not align with expectations, leading to tension, conflicts, and discomfort in relationships. Women frequently experience psycho-emotional challenges, including exhaustion, anxiety, irritability, insomnia, and relationship tension. They often lack the skills to address new challenges, making it difficult to navigate and resolve problems in unfamiliar circumstances [2, 3].

Regarding the psychological characteristics of veterans, it is essential to note that the process of reintegration after combat occurs in several phases [2]:

Phase 1: "Excitement" or "Honeymoon"

This phase lasts 2–3 months. During this period, veterans often feel euphoria at having survived, display self-confidence, and adopt an active life stance. Their "I-concept" is frequently shaped by the creation of a hero image, which may clash with the expectations of others, resulting in a "barrier of misunderstanding" and negative reactions. This dissonance leads to inner conflicts, often manifesting in the interplay of love (acceptance) and justice, accompanied by guilt, emotional ambivalence, social adaptation challenges, aggression, apathy, helplessness, regression, or substance abuse. Toward the end of this phase, symptoms of the next stage begin to emerge.

Phase 2: "Disillusionment"

This phase lasts 8–10 months and involves veterans re-evaluating their role in the war (the "Meaning/Future/Values" area). Adjusting to civilian life becomes challenging due to domestic, social, and moral difficulties. The hero image fades from their self-concept, often giving way to symptoms of post-traumatic stress disorder (PTSD), reduced self-esteem, reoriented values toward individual interests, and minimal social activity. During this crisis phase, all relationship spheres ("I," "You," "We," "Primordial We") can be negatively affected by re-assessing past experiences and personal concepts. Consequently, mental and physical health may deteriorate, with increased anxiety and frustration.

Despite these difficulties, certain resources can positively influence veterans' coping abilities: a stable financial situation (the "Work/Achievement" area), understanding from loved ones (the "Relationships" area), and the absence of disability (the "Body/Health" area).

Phase 3: "Recovery"

This phase begins 10–18 months after combat. Many veterans resume active life, but unresolved inner and actual conflicts hinder complete social adaptation. Veterans struggle with societal attitudes and self-perception, compounded by health and psychological challenges. This phase can extend for years or even a lifetime, as secondary adaptation to civilian life often remains incomplete.

The Role of Positive Psychotherapy

N. Peseschkian's principle of hope in Positive Psychotherapy offers a holistic approach to trauma, addressing both destructive consequences and the potential for growth [8]. This perspective shifts focus from pathology (symptoms, deficits, and disabilities) to recognizing abilities and potential. The principle of hope, through positive interpretation of experiences, encourages veterans and their families to find personal meaning in their lives.

According to N. Peseschkian [9, 10], discovering one's own meaning—created internally rather than imposed externally—transforms individuals from being passive objects ("It happened to me...") to active subjects of their lives. This sense of subjectivity instills hope for the future.

In this search for meaning, veterans and their families can effectively use N. Peseschkian's circle model, which encompasses relationships from the individual ("I"), to the family ("We"), to larger contexts such as the community, city, nation, and

humanity ("Primordial We"). Expanding identity in this way enables veterans and their families to better understand and integrate their experiences, fostering resilience and hope.

Returning to civilian life, servicemen often retain mental responses shaped during war, which become maladaptive in non-combat settings. Rules that ensure military effectiveness in combat, such as "No feelings, no emotions," "Stop in chaos," "React when threatened," "Trust only trusted friends," and "Always plan ahead" [2], can create difficulties in civilian life.

Applying the principle of hope and a positive approach in working with the military involves viewing these mental states as capacities rather than disorders, following N. Peseschkian's Positive psychotherapy model [10]. Behaviors formed in combat, such as vigilance or planning, can be redefined as strengths that help veterans adapt to peaceful life. Similarly, phantom pain may be seen as an ability to maintain body integrity through sensation, and sleep disturbances as vigilance in the face of ongoing threats. Reframing these symptoms as abilities emphasizes their functional value and aids in adaptation.

A transcultural approach is also invaluable in understanding and bridging the gap between civilian and military "cultures". By viewing them as distinct cultural groups, questions like "What do we have in common?" "What are our differences?" and "What can we learn from each other?" foster mutual understanding and improved communication.

The principles of Positive psychotherapy help address key challenges in assisting the military and their families, including stigmatization and the mutual alienation of civilians and military personnel [2, 4, 5].

The main goals of psychological assistance to servicemen, veterans, and their families are to: preserve and restore mental health; prevent the development of mental and behavioral disorders; help overcome life difficulties; mitigate the effects of traumatic events; prevent disability.

Psychological assistance includes: mental health first aid (support after traumatic events to reduce stress and promote adaptation), psychological diagnosis (assessment of psychological state, monitoring of mental health, identification of needs and methods of assistance), psychoeducation (prevention of mental disorders, formation of a general understanding of one's own mental health and skills to take care of it); psychological counseling (assistance in solving emotional and behavioral problems related to difficult life situations) and psychotherapy (intervention to address deeper mental health issues).

Based on the experience of organizing psychological assistance for military personnel and their families in Ukraine, such assistance can be structured into the following levels [11]:

1. **First Level**: Social and psychological support and services, including social adaptation, counseling, and general support.
2. **Second Level**: Psychological assistance.
3. **Third Level**: Comprehensive medical, psychological, and psychotherapeutic care.

From the perspective of Positive Psychotherapy, these levels align with three types of work:

- **Cognitive-Behavioral Level**: Social and psychological support to facilitate adaptation.
- **Conflict Level**: Addressing conflicts (actual, partially inner, or key) through psychological counseling.
- **Structural Level**: Psychotherapy involving deep work with trauma, re-traumatized experiences, personality structure, and basic conflicts [12].

The appropriate level of psychological assistance for military personnel, veterans, and their families is determined through psychological diagnostics, considering the individual's life circumstances and health status.

Non-specialists in mental health who have undergone relevant training can provide first-level psychological assistance [11]. These individuals may include members of veteran and disability associations, employees of healthcare and educational institutions, military registration

offices, social services, public and charitable organizations, and religious institutions.

This involvement of the social system, not just individual mental health professionals, contributes to the effective resolution of many actual conflicts, which reduces the likelihood of actualizing inner conflicts and provoking symptoms of disorders and mental disorders.

At the second level, psychological assistance is provided by mental health professionals and focuses on addressing problems caused by traumatic events (e.g., combat, captivity, crises, emergencies). This involves restoring a sense of security, stabilizing the individual, processing traumatic experiences, and reducing emotional stress. Preventive and therapeutic measures are also implemented for individuals and families experiencing stress or at increased risk of developing mental disorders. Counseling is offered to individuals without diagnosed disorders but at risk of developing them.

One of the most common reasons military personnel seek counseling is concern about changes in their mental state. Out of fear of potential consequences, they often avoid consulting doctors. Their primary question is, "Am I normal?" It is crucial for counseling psychologists to recognize the limits of their competence and refer clients to psychiatrists or psychotherapists when necessary [2].

Experience from Ukrainian psychologists working with the military indicates that positive prognosis for psychological counseling are associated with clients who are mentally and physically healthy, under 45 years of age, aware of their problems, adequately motivated to seek help, willing to take responsibility, possess adequate self-esteem, can handle stress without regressive or repressive reactions, and whose psychological issues are situational rather than chronic.

Third-level psychological assistance is delivered by a multidisciplinary team using a case management approach. This comprehensive method involves treating mental health disorders through a structured program that integrates psychological and psychotherapeutic interventions, as well as, when necessary, medication and physical rehabilitation.

The experience of Ukrainian psychologists aligns with the principles of Positive Psychotherapy, emphasizing hope, balance, and self-help, by showing that not all combatants require psychological assistance [2]. Macro- and micro-events of military experience do not always result in macro- or micro-trauma and can often be managed as conscious actual conflicts that individuals can resolve independently. Psychological and psychotherapeutic assistance is necessary for military personnel and their family members when their current situation (actual conflict) reactivates a basic conflict (stemming from childhood or previous traumatic experiences), leading to unconscious inner conflict, mental stress, and symptoms.

Research by Ukrainian positive psychotherapists working with the military reveals a connection between military resilience and nearly all primary actual capabilities. This suggests that resilience is supported by a strong sense of self-identity, belief in one's abilities, and trust in others, including capabilities such as tenderness, patience, trust, and hope [13]. Among secondary actual capabilities, loyalty—encompassing commitment to comrades, units, and the country—as well as honesty (straightforwardness), play a significant role in enhancing the resilience of servicemen.

Based on the data presented in the military, we find confirmation of the positive psychotherapy's statements about primary actual capabilities as the foundation of the personality structure, which is laid at the stage of attachment in early childhood in relationships with significant adults, as well as the role of the key conflict of directness politeness in a person's ability to resolve actual conflicts or, conversely, in the production of symptoms of disorders. In our opinion, it is the sufficient development of primary actual capabilities in balance with the actual capability of directness that constitute the prerequisites for a higher level of structural integration of the military personality according to OPD-II [14], which provides them with internal conditions for overcoming the adverse effects of the experience of combat conditions.

In the book "Nossrat Peseschkian. East-West: Positive Psychotherapy in the Dialogue of Cultures", answering T. Kornbichler's question "If there is a war and a powerful state organization oppresses a minority or kills everyone, or if it is genocide in general... Where do you see the 'undeveloped' in this case?", N. Peseschkian, the founder of positive transcultural psychotherapy, answers: "We believe that the capacity to love is not developed in these people" [15].

Analyzing the 10-year experience of Ukrainian psychologists and psychotherapists, we can conclude that Positive Psychotherapy as a metatheory, which is based on a holistic vision of a person in the unity of the Body-Mental-Soul dimensions, is the best suited to the tasks of working with the military and their families. This approach makes it possible to combine psychological/psychotherapeutic care with psychophysiological methods of treatment and rehabilitation, on the one hand, and cooperation with the chaplaincy institute, on the other.

The balanced model of Positive Psychotherapy is used as a basis for building a system of support for the military at different stages of their recovery and rehabilitation. For example, N. Misan, based on the balance model and the integrative philosophy of PPT, describes the experience of stabilization in working with the military during their treatment after injuries [16]:

1. "Body/Health" area: regulation of the sleep process with the help of relaxation exercises and exercises to deepen and improve the quality of sleep; stress relief and restoration of emotional balance using exercises from the Breath-Body-Mind (BBM) system [17] (Dr. R. Brown, Dr. P. Gerbarg, USA); relief of bodily and emotional stress using hypnotic resource exercises; progressive muscle relaxation according to Jacobson; equine-assisted therapy, etc.
2. "Work/Achievement" area: mindfulness techniques to switch thinking to the present; searching for and setting new achievable goals in activities; intellectual exercises to develop cognitive functions.
3. "Relationships" area: mastering the BASIC Ph (Mooli Lahad) Multimodal Model for Coping with Stress and Uncertainty, group sessions with discussions; training in communication skills and non-violent communication; creating new traditions and rituals; equine-assisted therapy.
4. The "Meaning/Future/Values" area: using art techniques to work with the future; discussing parables; rewriting parables in one's own way (according to V. Karikash); watching short motivational videos, including those involving the military; discussing and selecting favorite activities and hobbies.

The experience of using the five-stage model in the method of Positive Psychotherapy (Peseschkian, 1980) for group work with women whose husbands are missing has been successful. This model is structurally conducive to working with difficult cases. The stage of **distancing** helps to reduce pain and restore the participants' resources, while parables and metaphors help to work with emotionally painful topics. The stage of **inventory** helps women realize the peculiarities of their relationship with their husbands before the war. The stage of **Situational encouragement** using stabilization techniques makes it possible to transform tensions into resources. The stage of **verbalization** is devoted to identifying ways to maintain the participants' life balance and take care of themselves and their families during the period of uncertainty. At the **final stage of expanding goals**, participants share their goals using visualization techniques and letters to their future.

The philosophy of the meta-approach in Positive psychotherapy allowed us to consider and describe the work with clients' military topics through a psychodynamic vision of conflicts and the prism of a systemic approach. At the same time, one of the most important and valuable healing components of the process of Positive psychotherapy and counseling is the humanistic orientation of the method. It is the building of a relationship, contact based on the basic capacity to "love," empathy,

humanism, the vision of the nature and value of a person (as a "mine of precious gemstones"), socialization, that gives war participants and witnesses of these events a chance for healing.

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50 Positive Psychotherapy and Dealing with Sexual Violence, Rape, and Trauma

Enver Çesko

Introduction

Positive Psychotherapy (PPT after Peseschkian) is a psychodynamic, humanistic, and integrative approach with a semi-structured method of treating various problems, conflicts, disorders, and pathologies. It views sexuality as a primary capacity in the development of human personality [28]. Human sexuality represents the basic component in interpersonal relationships, married couples, and two people who express their love, respect, and well-being in creating social, emotional, and intimate satisfaction. Any deviation or effort by one party to fulfill their wishes contrary to this goal represents pathology in interpersonal relationships. Such behavior increases the likelihood of sexual violence, rape, mistreatment, and alienation, with potential consequences including post-traumatic stress disorder (PTSD).

PPT at its inceptions was not focused on the with the study and treatment of sexual violence, sexual abuse or rape and their consequences. But sexuality in the theory of PPT is presented as a primary capacity that builds, develops, and accompanies the maintenance of the healthy well-being of every human being. Any deviation in the form of various inhumane behaviors represents an abuse of basic human rights where the consequences of this form of mistreatment are undoubtedly of a severe trauma. Therefore, even the different approaches to the treatment of such disorders as a result of traumas experienced either from early childhood or various conflict situations, can be treated with approaches from the field of psychosexual psychotherapy [4].

PPT usually distinguishes two types of trauma: micro-trauma and macro-trauma, where the first appears in everyday life as a current event of an actual conflict, and the second that occurred in the past during life as an event of loss, psychological wounds, torture and suffering that have serious consequences in the presentation of the basic conflict [27, 28].

The origin and history of the appearance and occurrence of violence, rape, sexual abuse and their traumatic consequences, may be traced to war, where the various forms of inhuman mistreatment are aimed at humiliation, terrorizing and torturing the victim. The act of violence means forcing the victim to realize the goals and interests of the perpetrator.

The World Health Organization (WHO) defines sexual violence as: "*Any sexual act, attempt to obtain a sexual act, unwanted sexual comments or advances, or acts to traffic or otherwise directed against a person's sexuality using coercion, by any person regardless of their relationship to the victim, in any setting, including but not limited to home and work*" ([37] *pp* 2).

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_50

The consequences of this form of violence are physical, psychological, emotional, and social disturbances. Whatever the consequences are, the presence of trauma is evident where the mental health of the victim is disturbed and requires adequate treatment.

Some Historical Facts About Sexual Violence, Rape, and Trauma

Looking at the international conventions on human rights, the term of sexual violence and rape, are mainly emphasized in the war situations happening in last three decades. The war first in Bosnia and Herzegovina (1992–1995), and later in Rwanda (1994) and again Kosovo (1999), women, but not only, started to ask for justice at the International Criminal Tribunal. The International Criminal Conventions in their documents described sexual violence as "any attack on their honor, in particular against rape, enforced prostitution, or any form of indecent assault." ([9]).

In the war and other civilian conflicts, women are the most vulnerable groups because the women are mostly focused on protection of the family, that's why they are subjected to sexual and other forms of violence. Sexual violence was used as weapons of genocide against the morale of the population. Some facts are showing that different kinds of sexual violence and specially rape, are the "best" psycho-spiritual weapons that perpetrators are using to terrorize, torture and genocide the victims. "…The act of rape denotes the rapists' intention to construct their masculinity by attacking and humiliating enemy women, which become the symbol of the masculine nation. In this way, rapists aim to deconstruct the masculine pride of the enemy. That is why Bosnian men were often forced to watch the raping of their wives and daughters" ([21], pp. 1070).

During the civilian war in Rwanda where genocide took place, 49% of Tutsi women and girls had been raped [2]. In Bosnian war some facts are showing that between 20.000 and 50.000 Muslim women were raped from Serbian commandos [7, 10, 24]. Studies include women, girls, and some men, during the Kosovo war 900 documented cases of survivors from sexual assault, where 621 cases were sexually abused by more than one perpetrator, 135 of survivors were under age 18 years old, and 56 cases out of 900 victims were men. The majority of victims, 603 cases of sexual violence was done in villages where 35 of victims became a pregnant as consequences of being raped [19, 20]. Recently, in Ukrainian war (2022) researches done with refugees who witnessed bombings, killings, torture, and sexual violence have a higher risk of developing depression, anxiety, and PTSD than those who did not have such experiences where the symptoms of depression can manifest throughout the entire migration/refugee process [6].

Almost all survivors who had been sexually abused and raped during the war, "have been exposed to the overlapping processes of traumatization, stigmatization, marginalization, poverty, deprivation and exclusion in post-war Bosnia" ([21] pp. 1068).

The literature shows that sexual survivors have serious psychiatric and psychological disorders [18, 23], with high prevalence of PTSD symptoms with most reporting self-accusation to suicidal ideas [14, 22]. The victims of sexual violence and rape, suffers also from the psychological violence, where both survivors sexual and psychological violence are closely associated with each other because the sexual violence directly or indirectly injure the dignity and integrity of women specifically as a public stigma [11]. The most frequent psychological symptoms were nausea, vomiting, headache, sweating, palpitations, flashbacks, nightmares, lack of appetite, sleep difficulties, and muscle pains, and many of them still suffer from severe depression, anxiety, somatization, obsessive-compulsiveness, social phobia, PTSD, and sexual dysfunctions [13].

Common Feelings of Survivors of Sexual Violence and Rape

It is important to say that not all survivors of sexual assault perceive in the same way emotional responses and behaviors. The act of sexual vio-

lence and rape can be extremely traumatic that can cause big changes in everyday life. There is nothing abnormal, except the perpetrator's behavior, and all survivor's reactions, in this abnormal situation, are normal reactions to the traumatic experience.

Some of the common feelings are:

- Feeling of guilt—"*Did I do anything wrong*?", "*No one can understand me*", etc.
- Feeling of being "crazy"—"*I feel like I am going to be crazy*".
- Feeling of emotional shocked and numbness—"*Crying uncontrollably, withdrawing, claiming to feel nothing or overwhelmed*".
- Feeling of being preoccupied with traumatic incident—"*Nightmares, sleep difficulties, depressive thoughts, general anxiety, or lack of appetite*".
- Feeling of not being able to control the behaviors—"*Nervous, difficulties with concentration, attention and hypervigilant reactions*".
- Feeling of fear and dread— "*Fear from places, persons or/and situations that reminds to the traumatic incident*".
- Feeling of shame and self-blame— "*I feel shame what was happened to me and because of that I am blaming myself*".
- Feeling of being isolated— "*I don't want to be with others, I want to be alone*".
- Feeling of sexual concerning—"*How I can continue my sexual life after what was happened to me*".

Some other authors are describing stages of recovering sexual violence, rape, and trauma.

- Open up about what happened to you.
- Cope with feelings of guilt and shame.
- Prepare for flashback and upsetting memories.
- Reconnect to your body and feelings.
- Stay connected.
- Nurture yourself.

All these common feelings and reactions clearly manifest changes in the daily life of victims, developing symptoms of posttraumatic stress disorders as described in the DSM-5 and ICD-11 [5, 8].

Understanding of Sexual Abuse, Violence, Rape, and Trauma from the Perspective of Positive Psychotherapy

No **matter what** happens, all human beings are passing through their life **dealing with what** in Positive Psychotherapy are known as macro and micro traumas [26–28], where the trauma is positively interpreted and trying to **rebalance** life by **empowering healthy** growth and development. Sexual abuse, violence, and sexual rape are actions that lead a person to behave atypically (not normal) with the aim of humiliating, torturing and alienating the victim's personality, causing severe trauma in the four areas of daily life. The perpetrator of violence, is trying to "hit" the physical body of the victim, first causes physical wounds by using forms of pressure, coercion and torture to "submit" to the demands and wishes of the perpetrator, to cause fear in his psyche (mind), anxiety, and panic in order to be emotionally deprived from the social environment (social emotions) affecting the alienation of the personality in the spiritual aspect as a psychotic disorder (philosophy of life, state of mind).

Sexual abuse, violence, rape and trauma from the perspective of Positive Psychotherapy can affect in four areas of Balance model. Using the Balance model graphically this can be seen in Fig. 50.1:

Disruption of the balance in the four aspects of daily life (somatic, cognition, social and emotional behavior, and concepts of belief) causes multiple traumas with severe consequences known as posttraumatic stress disorders [5, 8]. These consequences can be permanent in the human cortex throughout life if these are not treated, although with their treatment these consequences can be diminished, its traces may not be forgotten. In this situation, the mental health of a trauma victim is disturbed almost in all areas of daily life.

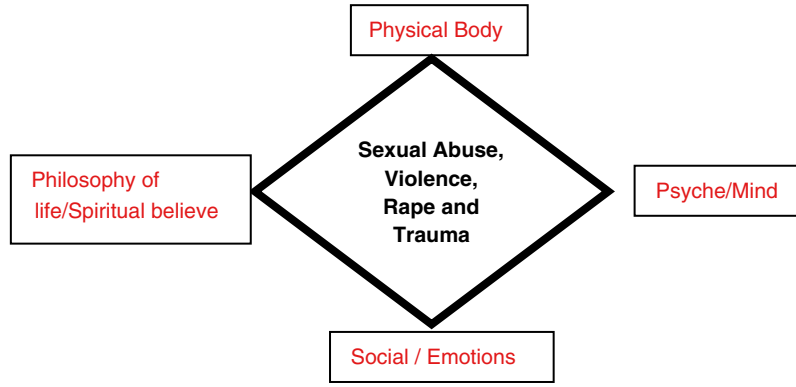


Fig. 50.1 Sexual abuse, violence, rape and trauma and impact to the four aspects of daily life

If we look at the four aspects of daily life, the victim has psychosomatic complaints in the body that are usually presented with body pain, where these pains are periodic in different parts of the body, as reactions inside and outside of whole central nervous system that usually is known as psychosomatic arc [17, 29, 32]—see Chap. 15, but are persistent at the same time. In terms of work and performance, there are complaints of difficulties in achieving success due to decreased performance, difficulties in concentration, keeping attention, forgetfulness and low motivation for work and achievement. Simply, functioning in cognitive processes is quite reduced to the extent of disorder. In contacts and relations, the victim has decreased motivation for contacts and keeping relationships, also due to the fear of stigma and the tendency to self-isolation. This condition usually leads the victim to fall into a depressive state where this condition, if is not treated, can provoke suicidal thoughts. And in the area of the future and meaning as well as the concept of life, the victim manifests signs and behaviors of anxiety, obsessive thoughts, hallucinatory ideas, and may develop psychotic disorders.

If we use the balance model in the four aspects of daily life, then it looks as shown in Fig. 50.2.

The first research done on the psychological effects of sexual violence and rape show the emotional and physical changes a few hours after the crime, where 96% of the respondents stated that they had fear, worries and had tremors in the body. 92% of the victims were terrified and confused, and after 3–4 h of the rape, 84% of the victims had depression, 96% were exhausted, and 88% of the victims felt powerless in their bodies [16]. Research also shows that 94% of victims for a short time develop the first symptoms that meet the criteria for diagnosis with PTSD [1, 3].

The psychological disorders that victims of sexual violence and rape have are among the most diverse, but the most frequent complaints are fear and anxiety, stress with traumatic disorder, depression, lack of self-confidence, social adjustment, sexual functioning, paranoid thoughts, and tendencies to psychotic behaviors. Such symptoms definitely develop many post-traumatic stress disorders with multiple implications in all aspects of daily life. There is no doubt that the experience of rape is a serious trauma for the victim and that this trauma has serious consequences on the mental health and overall well-being of the victim. This trauma, in particular, has disruptive consequences in the daily organization of the victim, in interpersonal relationships, in memory functions, as well as in the creation and development of vital concepts [12].

From the historical point of view, the word "trauma" was used in ancient Greek "injury" or "to plow", "to prick", or "to punch". This was used mostly by soldiers to express their injuries received from punctures in their armor [36].

Therefore, trauma is the body's reaction to an event where the content of the event has an impact on the entire nervous system and the entire human organism. Changes with numerous disorders in the nervous system and in the functioning of the organism, manifest with numerous symptoms in the four aspects of daily life.

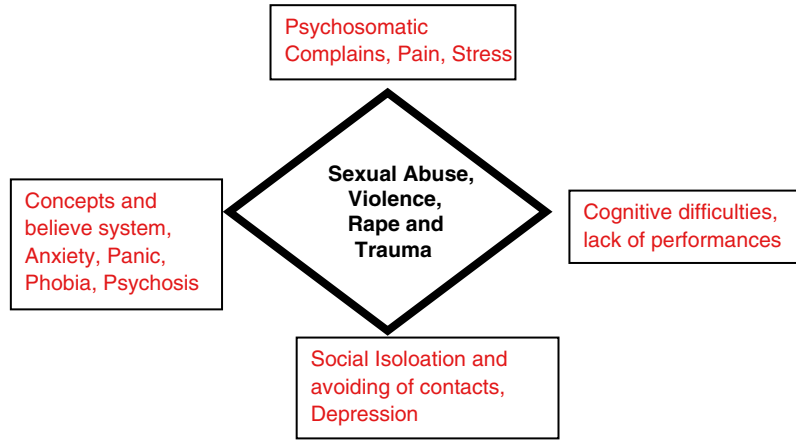


Fig. 50.2 Sexual abuse, violence, rape and trauma and possible mental disorders

Therefore, the events experienced and memorized in the cortex cause many changes in the dysfunction of physical, mental, emotional, social, and spiritual concepts.

So, *trauma as an experience processed after an event that has an impact on the central nervous system and the reactions of the central nervous system are influencing the entire human body, breaking the* homeostasis in human organisms. That's why the trauma is not what we had experienced but what we have in our body. New findings in neuroscience related to trauma show how our body is memorizing the traces experienced, keeping important scores. Psychologically people who have traumatic experiences, mostly suffer from grief and loss. During this process of mourning anxiety and depressive mood is often present which may continue for years or during long life, because of loss of limb and trying to adjust to the new "normal way". So, the *deep understanding of trauma is rooted in the dynamic process of conflicts, where trauma is presented as a conflict between stimulus (perpetrator) and response (victim).* Stimuluses are always triggering our nervous system and our self is responding to certain stimuluses immediately by freezing, fighting or escaping from reality, where sympathetic nervous system (SNP) is activating automatically and to stabilize the SNP it is need to work with polyvagal theory focusing on parasympathetic nervous system (PSNS) [31]. The consequences from this kind of reaction, through our body, are recorded in our cortex as a traumatic event where the trauma is a conflict between the situation, what is happening, on one side, and how the body is reacting to that traumatic situation, on another side. Hence trauma as a consequence of conflict consists of two main elements; situation and reaction. In this situation, two types of reaction are presented; attack and defense where attack is identified with the perpetrator and defense with the victim, there is no in-between.

Using the personal experiences of being in a war situation, if somebody is asked "why war happens"?, to be honest, there is no clear answer, nor a definition. The only thing that is clear for me is that people have a lack of love among themselves. Love is a kind of emotional energy that is integrated between people in how they want to treat each other and be accepted among themselves. Love between people and nations is seen in acceptance, respect, understanding, justice, and trust in the achievements, developments, and successes of people and nations with whom they cooperate. On the contrary, when love is absent and hatred, rage, greed, envy, and jealousy dominate, violence, torture, and death appear, then it is logical it is logical to have a conflict, and in the absence of a solution to the conflict, the emergence of war is inevitable. Therefore, the war is the destruction of love, where "tanatos" does not recognize the "eros", and all these can have severe traumatic consequences.

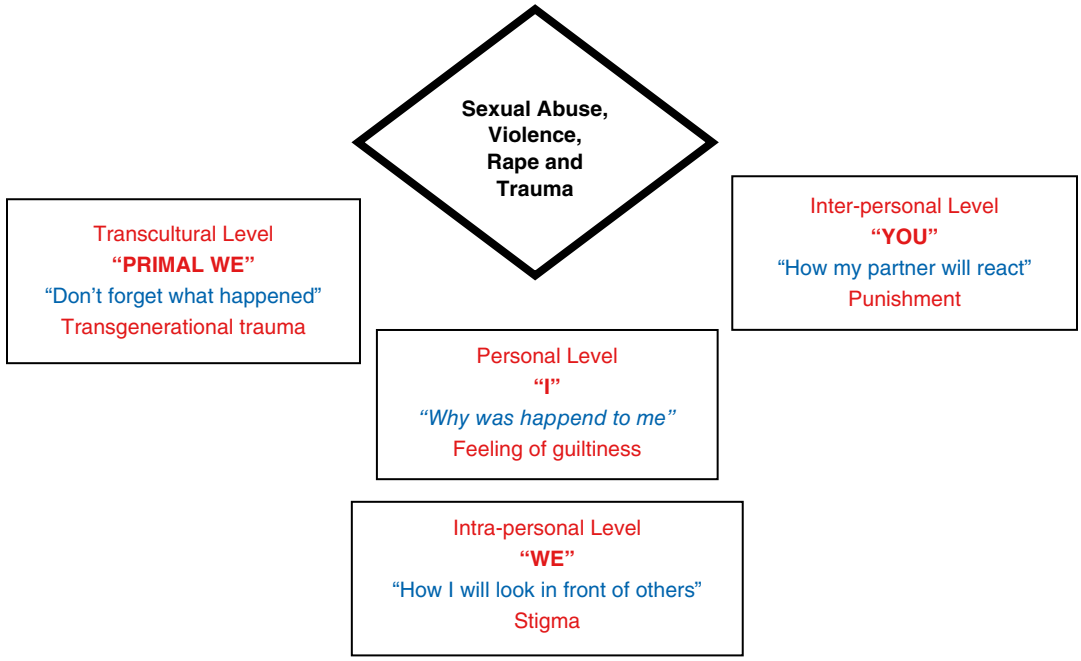


Fig. 50.3 Model dimension in sexual abuse, violence, rape, and trauma situation

In Positive Psychotherapy, a model dimension that represents finding the source of conflicts from the past to the present and awareness for the future, in cases of sexual violence, rape and trauma, can be seen by understanding the four levels as they are; the personal level, the inter-personal level, the intra-personal level, and the transcultural level.

On a personal level the victim is presenting **"I"** with myself, where usually comes the question: *"Why did this happen to me?"*. What the body is scoring are mostly pain and psychosomatic complaints, and in the senses, the victim has feelings of guilt. At the interpersonal level are **"YOU"** with my partner (each other) and the usual question is: *"How will my partner react to what happened to me?"*. From the transcultural aspect of view the common reaction is punishment of the victim, therefore, the victim in this case becomes both guilty and suffering. In the interpersonal level, how **"WE"** are in the society, means: *"How will I look in front of my family, friends, community, and society?"*. This is producing and strengthening the stigma in the general population. And in the transcultural level, where one finds the **"PRIMAL WE"** shows how the trauma may persist in the cumulative way known as transgenerational trauma. *"I will never forget what happened to us in the war?"*, is the common phrase that is mentioned, and this is developing the collective trauma. This can be seen visually in Fig. 50.3.

Survivors from sexual violence and rape, given the weight of their traumatic experiences, have a hard time making any positive interpretation of the experienced event. But the only thing that can be done is an attempt for a narrative integration of the experienced events. It can help the survivors to cope and manage in daily life with fewer repetitive thoughts and flashbacks.

Treatment of Sexual Violence, Rape, and Trauma According to Positive Psychotherapy

In PPT there are three basic principles that can apply in almost every case and with all clients. In the first principle, *Principle of Hope*, the role of therapist is to encourage the client to seek and to find hope for recovery, that not everything is dark

and ruined or over and done with. The second principle is the *Balance Model*, where the therapist works on the awareness of the client's capacities to balance the life energy in the four aspects of everyday life. The therapist works on the awareness of the client's capacities to balance the life energy in the four aspects of daily life. It is important to work on the re-integration of traumatic experiences by creating an efferent stability in the body, successes in work and daily activities, in social relations as well as in concepts, creating positive values for well-being. And the third principle is the *Five-Stage Treatment* which addresses all complaints that the victims suffered during their life experiences. The five stages of treatment are as follows *Observation with Distancing; Inventory; Situational Encouragement; Verbalization; and Broadening of Goals*. All these five stages are interrelated with each other during the whole therapeutic process where one stage can help to combine as preparation for development of another stage.

Stage One: Observation and Distancing

In this stage, the therapeutic process is focused on understanding the proper circumstances of what happened, how it happened, and under which conditions/environment the act happened. In this stage the therapist is giving opportunity for the client to re-organize self-reflection of the memories from the past experiences, and supporting to express painful feelings whatever they are, without any judgment and critics and trying to strength clients' strong actual capacities such are; hope, faith, justice, trust, honesty, time, contact, diligence, and obedience [28]. It is important to present the actual balance model in four areas of life as it is illustrated in Fig. 50.4.

Using stories often helps clients/victims to overcome and understand their psychological pain, feeling that she/he is not alone with what has happened. By reflecting experiences, and with guidance of a therapist, the client is expressing symptoms that are most affecting in four areas in the balance model. "The positive attitude of the therapist provides the patient with the alternative psychodynamic experience of the symptoms: *curiosity vs. avoidance, understanding vs. shame, developing capacities vs. anxiety, hope vs. despair*" ([17] pp. 170).

Stage Two: Inventory

In the Inventory stage, the therapist is focusing on the current relationship with family members who are closed to her/him. That's why it is important to use the Differentiation Analytic Inventory (DAI) in the sense of rebuilding individual resources and coping strategies, which is a kind of psychodynamic modeling. This is when the client is trying to attribute psycho-emotional values and needs in the perspective of fulfilling the elementary satisfactions in the safety environ-

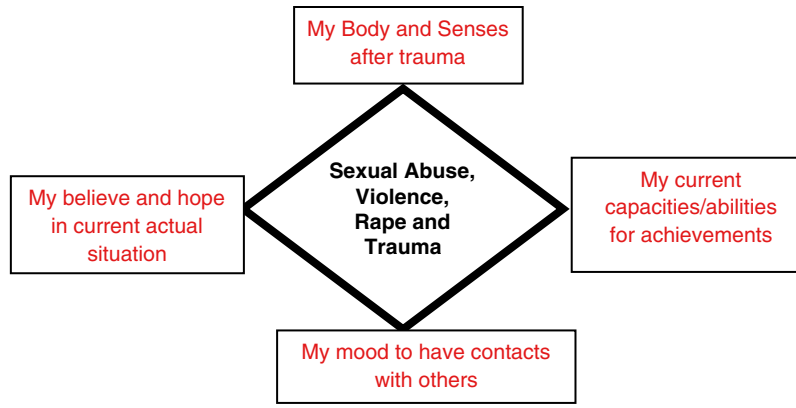


Fig. 50.4 Balance model in four areas of survivor from sexual abuse, violence, rape, and trauma

ment. In this stage, it is necessary to involve model dimension as was mentioned above in Fig. 50.3. To model means identification with a figure, the imitation of role aspects, the adoption of desired characteristics, and the cessation of undesirable ones. Aside from positive identification, there is also the possibility of using a "bad model" as a warning or as the stimulus to try other alternatives ([27] pp. 160). In this stage, the therapist is investigating actual capacities to see how the client (survivor) is able to understand the basic conflict and to cope with actual conflict, and to bring the frame in the safety place how can raise the clients' hope and trust to the process of treatment to rich the post-traumatic growth [35].

Stage Three: Situational Encouragement

During the therapeutic process, the most important thing is what questions are asked to the client in such a way that they do not provoke the traumas experienced, on one side, and do not violate the integrity of the client, on the other side. Therefore, the use of stories that can help the client find a positive meaning for the trauma experienced, as a kind of supporting mechanism to encourage the continuation of daily life, which means the use of healthy mechanisms to return to reality. Using the stories ("Iron is not always hard", "Fifty years of politeness", "Courage for Truth", "Give Him Your Hand", etc. etc., [25] can help to get away from "repetitive thoughts", not only the survivor memories, but also the survivors' family to understand the circumstantial situation unchangeable trait, and depended on the situation and on the survivors' and his family itself [27]. The main thing that is required from the therapist is to work on finding a healthy perspective in strengthening the relationships between couples, the entire family, and the social community. This means that both sides (survivor and family members of survivor) are learning how to distinguish between feelings being offended or desecrated of honor and shame, justice and trust, or love and faithfulness. That's why this is called a "psychoserum" to relativize the values and existing integrity. Work with trauma has to be focused on feelings and emotions first and then to actions and concepts. There are four emotional needs that have to be worked on. Figure 50.5 illustrates the four emotional needs that are important in the treatment process, where all these four emotional needs are bonded with each other to help in better understanding the trauma experiences and creating the opportunity to cope with past experiences.

"Regardless of what stresses and disturbances afflict us and cut down our ability to achieve—of which we are so proud—we can always fall back on our basic existential capacities to love and to know, and, starting from them, seek realistic new ways which are more in accord with our current possibilities" ([26] pp. 67–68).

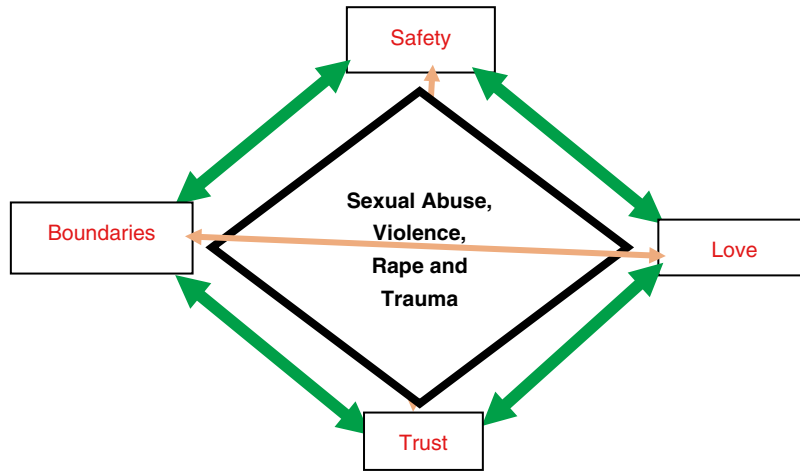


Fig. 50.5 Work with four emotional needs during the treatment of survivors from sexual abuse, violence, rape, and trauma

Stage Four: Verbalization

After strengthening and relativizing values and concepts of how the traumatic experience happened, by using the four emotional needs, than it is need to create a healthy atmosphere by improving the survivors' ability to understand her own feelings about what's happened and reactions how to move forward to cope with past trauma by strengthening their own defense mechanisms. In this stage the therapist has to come back to the basic conflict to emphasize if the client (survivor) understood her past trauma and to continue to be part of her own family and social engagement how to deal with current actual conflict to prevent her from suppressing inner conflict so not to provoke any psychosomatic symptoms. Therapists have to be aware that if the client (survivor) is not able to deal with her actual conflict, it may provoke and activate the key conflict from guilty feelings as a result of suffering from traumatic experiences. It is important to ask the client (survivor) how she/he sees themselves enough recovered and strengthened to move and overcome from trauma experienced and start to go further in daily life activities without asking to forget the situation and what happened. As a technique in this stage we can use "**dialog with my past trauma**" for the purpose to express the deep suppressed feelings or to use the "**empty chair**", modified gestalt technique [30].

Stage Five: Broadening of Goals

This is a stage when the therapist is focusing on the future plans and re-finding the meaning of life and starting to overcome trauma and to find new perspectives and setting new goals for certain years. It is important to give the space for mutual negotiation between the survivor and survivors' family members on how together they can move and support each other. The focus is on rebuilding the partner relationships and learning the lessons for self-help by comparing the past balance model from the stage of Observation/Distancing with the desired balance model in the stage of Broadening the Goal. The therapist asks the client, together with the partner, to present a balance of the common model of how they want to have it, by comparing it with what was presented in the first stage of treatment, which was presented in Observation/Distancing. This is illustrated in Figs. 50.6 and 50.7.

Here is a space to develop and strengthen new ideas, opinions, and concepts to have a healthy future in a realistic perspective. The questions as they are; "*from where I came*", *"what I experienced", "how I survive",* and *"where I am now", "how I am doing now",* and *"what I would like to be and what to do",* are most frequently questions that therapist has to be reflected and to overcome through the last ending session. After all these kinds of lessons are learned, therapist and

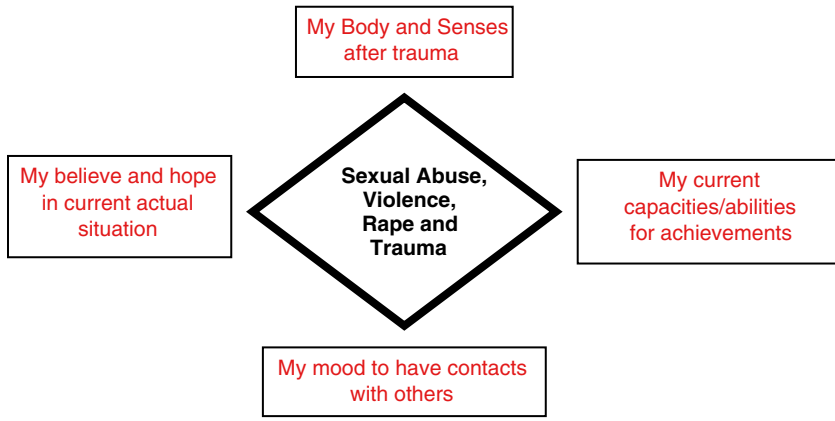


Fig. 50.6 Victim's Balance Model as was presented in Observation/Distancing stage

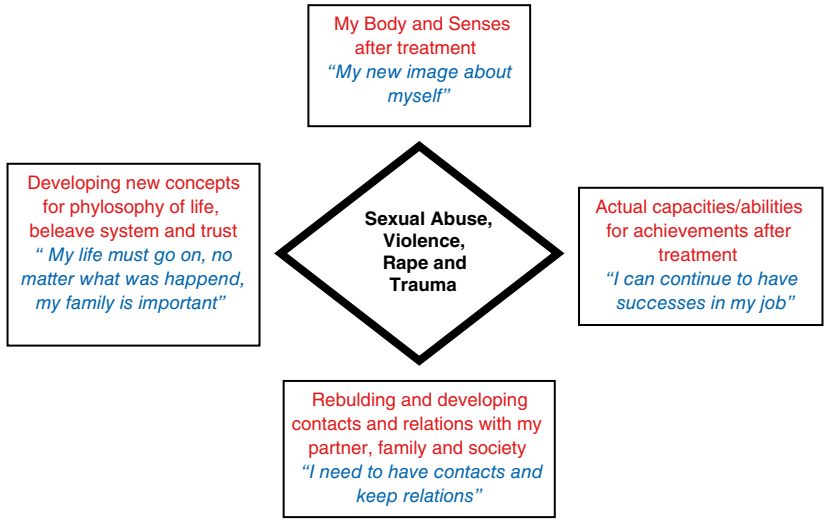


Fig. 50.7 Balance Model of partners after treatment in stage Broadening the Goals

client are deciding how they want to move on and to find mutual reflections about ending the treatment process.

Conclusion

Persons who have survived sexual violence and rape with their traumatic consequences, very commonly present numerous symptoms of post-traumatic stress disorders (PTSD). Circumstances and situations that victims have experienced of sexual violence and rape have special characteristics that also affect the treatment of the consequences to restore balance in mental, emotional, social and psychosomatic health. The mental health of victims who had experienced war trauma, "triggered the spiral of violence and multiplied the psychological, economic and social violence" ([21], pp. 1074). For this reason, it is important that the victim and the whole family should have adequate support and treatment from the consequences of the traumas experienced, so that the society and the whole community have a healthy well-being. Treatment of the consequences of trauma from sexual violence and rape, is a process that includes complex and long-term treatment, because the nature of sexual violence and rape is complex, therefore the treatment process requires patience, commitment, trust and hope for a better healthy life. Using the balance model and the whole methodology from Positive Psychotherapy in healing the trauma of survivors from sexual violence and rape, is one of promising model with potential applicability in variety situations, cultures, and circumstances.

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51 Transgenerational Trauma with Positive and Transcultural Psychotherapy

Ewa Dobiała and Ivan Kirillov

Introduction

The study of the impact of past traumatic events on the development of personality structure, survival strategies, resilience factors, and secondary clinical consequences for subsequent generations has been a subject of interest for many researchers, including Volkan [1], Yehuda [2], Kellermann [3], Schutzenberger [4], Hollis [5], Apprey [6], Boszormenyi-Nagy [7], and Faimberg [8].

Charles Barber, an American psychiatrist, described the modern era as the "age of trauma." Current research conducted on the Polish population shows a similar level of PTSD prevalence within the studied group to that observed immediately after World War II [9]. This is a consequence of the long-term social effects of historical collective trauma, which still has not been effectively incorporated into the clinical interest area [10].

Canadian clinicians Rakoff et al. [11] discovered that previous psychotherapeutic approaches used to treat depression, grief, and trauma responses were ineffective for Holocaust survivors. This discovery initiated intensive research on the impact of World War II events on survivors and their descendants, as well as on society and the world as a whole. Despite attempts to theoretically develop therapeutic approaches and their implementation in various psychotherapy models, the topic of transgenerational trauma and historical collective trauma remains on the margins of clinical understanding and social awareness.

The impact of transgenerational trauma [12] and historical collective trauma on subsequent generations can be profound and long-lasting. Children and grandchildren of those who experienced trauma may exhibit elevated levels of anxiety, depression, difficulty forming and maintaining relationships, and emotional regulation problems often leading to behavioral disorders. Awareness of these phenomena is crucial for effective therapy and support for those affected by the described dynamics. Social understanding of transgenerational trauma's essence, transmission mechanisms, and the implementation of appropriate therapeutic strategies is necessary to break the cycle of trauma and improve the quality of life for future generations.

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_51

Definitions and Key Concepts

Resilience Factors A set of resources, traits, and strategies that enable an individual to effectively cope with difficulties, stress, and potentially traumatic events, thereby sustaining mental and emotional health. A core determinant of resilience factors is the level of development of primary capacities and the degree of integration of the personality structure (see autism chapter). According to the concept of Positive Psychotherapy (PPT after Peseschkian), resilience factors can be described in four areas of the balance model (Chap. 8 Peseschkian, H., Remmers, A., Life balance with Positive Psychotherapy):

- Relationship Area: the ability to build and sustain rich interpersonal relationships by means of the primary capacities of*: love* and *care*.
- Achievement Area: The ability to act to achieve results, meeting primary needs, adjusting and optimizing the efficiency of action algorithms in changing realities by means of the primary capacities of: *trust* and *time*.
- Imagination Area: The ability to find meaning (causality) in inner and outer experiences, to extend a reality through imaginative experiments, and to define values by idealizing what best meets one's vital and primary needs. The use of assumed meanings to predict the consequences of—and provide values for—decision-making gives one the feeling of control, i.e., an inner sense of agency, accessing the primary capacities of: *meaning* and *ideal*.
- Body Area: The ability to perceive inner and outer physical experiences allows individuals to consciously identify their own and others' states and emotions, and thereby hypothesize needs through the primary capacities of: *contact* and pleasure.

Window of Tolerance A concept in psychotherapy and neurobiology that describes the optimal range of emotional arousal within which an individual can effectively function and process information. The size of the window of tolerance is influenced by active resilience factors. Emotional arousal exceeding the window of tolerance overactivates the sympathetic nervous system (fight or flee response) and overloads it, followed by activation of the dorsal portion of the parasympathetic nervous system, resulting in freezing and dissociation (trauma).

Trauma A response to an event or series of events that have a significant negative impact on an individual's psyche, often leading to long-term emotional and mental consequences. The dynamics of trauma formation are related to the phenomenon of freezing and dissociation, causing post-traumatic personality fragmentation into areas that remember the trauma, serve survival, and remain healthy (before the traumatic event occurred) [13].

Dimensions of Trauma Trauma can be experienced in intrapsychic relationships with Self (I), in interpersonal relationships with partners (You), with family and other social groups (We), and/or with the collective unconscious and consciousness of culture and worldview (Primary We). The more dimensions of relationships that are affected by trauma, the deeper its individual and transgenerational effects (e.g., war trauma, Holocaust, etc.).

Transgenerational Trauma The transmission of trauma effects from one generation to another, where subsequent generations experience the negative effects of their ancestors' trauma, even though they were not direct witnesses.

Historical Collective Trauma Refers to complex trauma experienced by a group of people sharing a common identity, affiliation, or circumstances and is experienced over many generations. The term initially described the experiences

of the children of Holocaust survivors [3]. It now applies to numerous indigenous groups worldwide and other communities experiencing oppression and mass trauma. Historical trauma includes three key elements: injury, shared experience by the group, and generational span, meaning contemporary members of affected groups may experience trauma-related symptoms even though they were not present during past traumatic events.

Transgenerational Transmission The process by which values, beliefs, behaviors, and traumatic experiences are passed from one generation to the next. This phenomenon occurs on multiple levels:

1. At the level of emotional availability to bond with a child – phenomena of dissociation and post-traumatic strategies affecting an individual's defense mechanisms directly influence the attachment style and the risk of forming attachment trauma in the next generation [13]. The development of basic potentialities occurring in the relationship with a caregiver is hindered in children of traumatized parents.
2. At the systemic level—through customs, traditions, and ceremonies shape survival strategies that are passed from generation to generation. These influence the development of primary capacities in the child's experience of: a parent's attitude to them (I), the modeling of a parent's interpersonal communications (You), social interactions (We), and relationships with the collective cultural context (Primal We).
3. Possibly at the epigenetic level – through the activation of mechanisms affecting gene expression such as chemical modifications of DNA (methylation), changes in chromatin structure (gene availability for transcriptional processes, affecting gene expression that can be inherited), and impact on RNA (influence on microRNAs regulating gene expression at the post-transcriptional level). The above-described phenomena require further research [14].

Application—Consultations with Residents of a Small Polish Town Affected by Fentanyl Deaths

To illustrate the potential consequences related to historical collective trauma and the intergenerational development of intrapsychic, interpersonal, and systemic dynamics that may influence the development of another collective trauma, a case description of consultations concerning a small town in central Poland, taking place in June 2024, will be presented.

The consultations were prompted by the deaths of over 100 young residents, aged 20–40, in a town of 8500 people, caused by fentanyl overdoses. For over 20 years, bodies of young individuals who did not wake up after taking the drug have been carried out from basements, staircases, and apartments. This situation persists, and affects young people across all levels of education and social groups.

Residents do not discuss this issue in family or neighborhood conversations, and funerals are conducted in silence. The families of the victims often conceal the true cause of death, with "cardiorespiratory failure" frequently cited in medical documents. Relatives and acquaintances are commonly told that the young person "died of a heart attack." Despite the large number of victims, the issue has not been publicly addressed until recently. Responsible services and city organizations have not taken effective action to change the ongoing situation.

"*It was trivialized, no autopsies were conducted for drug detection, and events were classified as natural deaths.*"

Observation

The observation stage in positive psychotherapy (PPT) is when the client's story is presented for the first time in the consultative/therapeutic space. The event causing the consultation may be related to the actual stressful situation that frustrates the client's value system, or to a persistent symptom whose cause usually lies in the past.

During the first meeting with consultants Adam Ejnik (town resident, local teacher, and journalist) and Ewa Dobiała (psychiatrist, psychotherapist, supervisor), feelings of helplessness, despondency, and hopelessness predominated among the town's residents. The current conflict brought by the community was the belief that city authorities had failed for many years in the face of the deadly threat faced by the young generation of residents. Attempts to act were suppressed, and those who dared to raise the topic in the public space were met with social ostracism and intimidation. The frustrated social norms (Chap. 2 secondary actual capacities, see chapter: Positive Psychotherapy: An Introduction) were reliability and justice. Residents felt that the city as a whole could not protect young people from the dangers of drugs and addiction. They expected actions to be taken and the acknowledgment of the problem's existence. The dominant defense mechanisms among both residents and officials were dissociation, denial, and/or negation. Most residents remained passive and unable to engage in open discussion about the situation.

"*The topic of drug problems in our town, in my opinion, has been present for over 20 years. I believe that during this period, no effective fight against addiction, related crime, and anti-drug prevention was undertaken. It seems to me that the problem is being trivialized, and the centers that should do so do not devote due attention to it. (…). This year, one of the mothers of a drug victim sought help from the editorial office of the popular Polish TV program,* TV Attantion! *It sparked a reaction, and the problem of the town began to be highlighted in the media. The police and city authorities also took action. The police strengthened patrols, and investigators from the provincial headquarters arrived. Several people were detained. The city introduced anti-drug programs. I feel, unfortunately, that everything will soon return to the way it was. There are already voices that the problem was artificially exaggerated by the media, that there is no problem, or if there is, it is no greater than in other towns. In my opinion, the town needs help. (…) Without such actions, in my view, the trauma will not disappear. It will become quieter and quieter until only whispers remain in the town, and young people can quietly die again." (Adam Ejnik, town resident).*

Contract

A four-hour consultation meeting was scheduled, with several representatives of the town's residents from different social groups and generations accepting the invitation. The reflecting team consisted of seven psychotherapists/supervisors from Poland (E. Dobiala, L. Lipman, K. Marquardt, R. Mizerska), France (Ch. Winkler), Ukraine (L. Moskalenko), and the USA (E. Fogelman) representing humanistic, psychodynamic, systemic, and transcultural approaches.

Inventory

The inventory stage of PPT allows a deeper understanding of the conflict, or symptom dynamics, by examining, among other things, history, dimensions of modeling and relationships (see Chap. 2: Positive Psychotherapy: An Introduction), and recurring patterns and areas covered by oblivion. These may indicate particularly difficult areas that require consideration of dissociation, denial, or negation phenomena in understanding intrapsychic and systemic dynamics.

History of the Town

The described town has a rich and painful history. It obtained city rights 200 years ago. Before the war, it was a Polish-Jewish town. Residents recall it as poor and neglected. The pre-war community was called "barefoot Antons" by the surrounding area due to prevailing poverty. At the turn of the nineteenth and twentieth centuries and in the interwar period, many families, both Polish and Jewish, emigrated to North and South America for economic reasons. During the interwar period, the town was deprived of electricity, and the streets were so neglected that, according to residents' stories, "horses and carts drowned in the mud flowing through them."

World War II left a huge mark on the town:

- In November 1939, the Jewish community, accounting for about 50% of the town's population, was expelled by the Germans. Over 95% of them perished in ghettos and death camps, particularly in Auschwitz-Birkenau.
- In January 1940, the Polish portion of the town, which had chronically or reactively relied on the support of a local psychiatrist due to the ongoing war, occupation, and the level of wartime cruelty, was targeted for murder by the Germans. These actions were part of a project implemented by the Third Reich throughout occupied Poland, aimed at "eliminating life unworthy of life" (*Vernichtung von lebensunwertem Leben*).
- In March/April 1940, representatives of the town's intelligentsia and clergy were deported by the Germans and later mostly murdered. Their first destination was a transitional camp in Działdowo (a covert extermination center), established to "discreetly carry out the necessary liquidation of Polish resistance activists" [15].

As a result of war crimes, the town lost over 60% of its inhabitants. After the war, the town's population was supplemented by migrants from nearby villages. The topic of wartime trauma was almost completely taboo. An unspoken agreement not to discuss the history of German occupation or the multi-ethnic origins of the native population was adopted. A small number of contemporary residents are aware of the Polish-Jewish roots of the town [16]. Other traumatic events concerning the fate of local intelligentsia and the mentally ill remain almost entirely forgotten. The conscious trace of them remains in the families of a few survivors and still limited historical studies.

Older residents recall the communist years as a time of some peace, despite openly referring to it as further Russian occupation. The scope of Soviet crimes: the victims of the NKVD camp established on the basis of the German transitional camp in Działdowo, deportations deep into the USSR, and other communist system repressions require further research. Most communist crimes were concealed and covered by Soviet propaganda. After the political changes in Poland in the 1990s, the town declined again, failing to find its own response to the challenges of emerging capitalism.

In the residents' statements, several themes were repeated:

- The belief that it is not possible to talk about difficult topics in the town.

"*There is a belief that we cannot talk about difficulties.*"
"We (in the family) were always on the side, we didn't analyze much, we look at problems shallowly, which allows us to live more easily."
"Difficult topics in our town are swept under the rug."
"At home, I learned that difficult matters are not discussed."
"I was surprised that outside our town, there are people who talk about both easy and difficult topics."

- A sense of shame about coming from this place felt by some current residents, mainly experienced in youth.

"*I remember being in high school and feeling ashamed. Subconsciously, I didn't know why, but that feeling was strong.*"
"I felt it subconsciously, I didn't want to admit I was from here."

- The belief that there is no future in the town and that leaving is the only way to be saved.

"*I wish us all to leave here as soon as possible. Escape is the only way.*"
"*You just have to leave here.*"
"*There is no hope here.*"
"*Nothing good awaits young people here.*"
"*I don't see a future here for my son.*"

- The belief that a well-educated person poses a threat, both to the town and to themselves.

"*A young, ambitious person with a degree returns to our town and is pushed away as far as possible.*"
"*Educated people are a threat to the existing arrangements in the town.*"

Situational Encouragement

At this stage, finding the positive function (see chapter: Positive psychotherapy: An introduction) of the reported problem becomes crucial. the symptom is usually the best possible response generated by the system to maintain fragile balance.

At this stage of the consultation process, residents noticed that history avoidance secured their collective self in hard times yet staying with their history within the closed space of their town makes it very difficult to bring about change. The strength of the symptom drew the attention of people from outside the town.

"*The world can help us.*"
"*We want to look broader; we want to understand.*"

Verbalization

The verbalization stage allows a traumatized group to bring the awareness of deeply rooted beliefs, emotions, and needs to the consciousness/semantic level. this stage provokes the emergence of connections between the past and the present, with intrapsychic, interpersonal, and systemic dynamics.

The experiences of residents over generations have reinforced the sense of existential threat and mistrust. For survival, during both the German occupation and communist repressions, the community had to hide emotions and the most difficult stories about both their relatives and the local community. Breaking this unspoken rule could result, depending on the decade, in being sent to a camp or prison, in deportation, or death. Despite the political system changes in Poland in the 1990s, the mechanisms established and transmitted through generations did not change. Even in the face of the deaths of over 100 people from the young generation, residents have remained in denial and negation, unable to break the established strategies that previously served survival. The situation has been complicated by the shame and fear, established after the German action "elimination of life unworthy of life," which arose from experiencing mental disorders among relatives. Since World War II, the town has had no professional facility to turn to for people experiencing mental suffering. The stories of people with psychiatric disorders and addictions have been hidden by families for decades.

During the ongoing consultation, residents began to connect their actual problems with the town's history. Statements such as the following emerged:

"*I wasn't aware of the role of history before, as if we discovered something; maybe the mothers of those who passed away could feel relieved that the death of their children wasn't their fault.*"

Residents started noticing the lack of historical and social narratives of the town history:

"*There is no sense of history in our town at all. Certain things aren't talked about. There are no traces.*"

"*There are no traces of the synagogue, not even commemorative plaques, nothing.*"

The reflecting team noted the dynamics of broken social and family bonds in the town:

"*I hear a lot of stories about broken bonds: between generations, within families, what social ties look like—bonds are broken between social layers, between people and authority.*"

Broadening the Goals

The stage of expanding goals involves defining new, achievable perspectives and objectives, utilizing the lessons learned from those experiencing transgenerational collective trauma to help the community confront the symptoms and its effects. At this stage, the specific possible steps to healing can be planned.

During the consultation, it was established that it is important to speak openly about problems and connect different perspectives to understand the deeper roots of the issues.

"*It's important to voice and discuss it. We need to connect the discrete dots and think about what we can do for students and youth, so they find pride in living here, so they can boast about being from this place.*"

Residents noted that merely arresting drug dealers would not solve the problem.

"*Catching dealers won't solve the problem. If there's no fentanyl, there will be other drugs. The issue is not the access to substances but the reason why these substances are used.*"

Further Discussion and Proposed Actions

- Joint Actions and Community Involvement: It was determined that various institutions and communities must be engaged in the process of healing the actual trauma and reactivating the town's historical collective wounds. Cooperation between these organizations is crucial for effectively addressing the issues.

  "*We need to involve different institutions in collaborative work. We can show the town authorities how we, as outsiders, can help those dealing with this problem.*"

- Open Discussion of Problems: It's important to break the silence and start openly discussing issues affecting the community.

- Working with Grief and Restoring Hope: During the consultation, the significance of working with grief and loss was emphasized. Addressing suppressed, unprocessed emotions is essential for the community to begin the healing process.

  "*It's about facing loss, working with grief over what was lost: perhaps loved ones, perhaps a sense of security. There is a need to restore hope that there is a way out. If this is collective trauma, the group needs to come together to empower themselves collectively. To open doors and let the light in.*"

- Education and Historical Awareness: The importance of education and historical awareness was also emphasized. Engaging with the town's history and its potential impact on the actual situation can help in understanding and healing trauma.

  "*Understanding the town's history is important to building people's awareness of where they come from and to what they belong. Social attempts at self-healing may be unfortunate, but what part of the community is doing, trying to highlight the history, may offer a second, healthier proposition, focusing on bearing witness. Here, in this place, many people lost their lives prematurely; how can we admit this?*"

- Openness to Change and External Collaboration: Meeting participants also stressed the importance of openness to change and external collaboration. Contact with external experts and institutions can bring new perspectives and solutions.

  "*I'm glad that committees are forming, that the town has started talking about it. It's important to hear an external voice, to step outside our patterns. These meetings are important; publicity is important.*"
  "*I'm happy that the world wants to help; we need this help because there never was any before.*"

Discussion

Reflecting on the actual mental condition of humanity, the impact of transgenerational trauma, historical collective trauma, and seeking factors that promote healing, a few psychotherapists participating in the Reflecting Team during the consultation shared their thoughts:

Eva Fogelman (PhD, licensed psychologist, author, filmmaker, pioneer in developing groups for generations of Holocaust survivors, New York, USA):

"*In the aftermath of World War II, many displaced Holocaust survivors found refuge in Displaced Persons Camps in Germany, Austria, and Italy, which lasted from 1945 until 1951, with some rare exceptions. Among them were also non-Jewish refugees. This post-liberation period served to rehabilitate people who had experienced massive psychic trauma and multiple losses. Many young adults who survived married and gave birth to children. Young children who had lost their childhood were given an opportunity to get an education. Fragmented selves were slowly working on integration. What was most healing was being with others who experienced the same hell and speaking to each other. When survivors moved on to a permanent destination, they joined existing fellowship organizations (lantzmanshaft) from their hometown. Others joined groups according to how they survived the war (e.g., Warsaw Ghetto, partisans, Lodz Ghetto). Meeting with others was like being in a self-help group. When speaking with others who share your hellish experience, one feels understood. Also, sharing behaviors that may seem extreme becomes normalized when others report similar behavior. Participating in these groups is like an extended family to those who lost most family members.*"

Lucyna Lipman (psychotherapist, supervisor, Polish Psychiatric Association, Polish Ericksonian Institute, Łódź, Poland):

"*Participation in the reflecting team during the meeting with the residents of the Mazovian town triggered many emotions, associations, and reflections in me. Although residents mainly talked about the present, the assumptions for which we gathered made it clear that we were meeting because of current tragic events, the causes of which we seek in a rather distant past. Our assumptions likely triggered associations, and in my case, they related to both personal and family experiences, as well as to areas of scientific exploration and psychotherapeutic and supervisory practice. The sentences spoken by the residents recalled the statements of survivors, read comments, and scientific reflections. It would be difficult to distinguish whether the unconscious content of the speaker's transgenerational transmission was reflected in the resident's statements, or whether both unconsciousnesses met and attempted to invite the richness of their shared resources to lift future generations from the abyss of their grandparents' dark experiences, freeing them from the compulsion to seek solutions, each of which further drew them into the energy of trauma. The unconsciousness of the adult residents, immersed in the town's dynamics, and the invited specialists from outside, sought constructive, shared resilience that would allow respectfully letting go of the past and turning towards building a fruitful future.*"

Renata Mizerska (psychotherapist, supervisor, psychologist, trainer; President of Polish Council for Psychotherapy - Federation of Associations, Advisory Board Member and Former President of the European Association for Gestalt Therapy, Member of the Accreditation and Certification Committee and Former President of the Polish Association for Gestalt Psychotherapy, Vice Chair of the European Training Standards Committee of the European Association of Psychotherapy; Warsaw, Poland).

The meeting was a deeply moving experience in which sadness and fear were accompanied by hope. I felt that the problem the community is struggling with is a phenomenon of trauma that has been repressed for generations — a trauma that has not found expression in words, relationships or even in the public space of this city. The silence that surrounds these traumas (WW2 and others) reinforces their presence and transforms them into other forms of expression, such as addiction. This addiction becomes a dominant figure in the relational field of the community and manifests itself in a sense of hopelessness, despair and fear that permeates the daily lives of many inhabitants. From the perspective of the humanistic-experiential approaches, including Gestalt and EFT, I see the creative adaptation of residents changing the way they are contacting - with themselves and the environment in order to cope with emotions that are too overwhelming to integrate. In this context, fentanyl becomes a tragic form of adaptation. It suppresses embodied memories that are too painful to be consciously and fully experienced.

Our role is not only to help the community cope with the phenomenon of addiction but also to give voice to what has been silenced and integrate what has been fragmented. From the field perspective, addiction is not just an individual problem, but an expression of a broader societal dynamic in which historical and social forces shape individual experiences. The emerging willingness of residents and local politicians to confront the phenomena of the past can lead to the closure of various figures and the uncovering of the primary adaptive emotions necessary for healing, opening space for co-

creation and the process of building a common ground for community growth.

Liudmyla Moskalenko (PhD., positive psychotherapist, supervisor of the Ukrainian Union of Psychotherapists, Head of the Board of the Ukrainian Institute of Positive Psychotherapy, President of the Ukrainian Association of Positive Psychotherapy, Poltava, Ukraine.)

I have been working with clients on the topic of transgenerational trauma for many years. The most important thing for me in this work is to maintain the "positum" vision based on positive psychotherapy: what abilities, healthy forces and potentials of the individual are behind the mechanism of transgenerational transmission of trauma and the symptoms that are now lived? And I try to see these abilities and opportunities at different levels, described by N. Peseschkian: for the client, for the family, for communities of people, for a locality, for a country and for humanity as a whole. Without this, transgenerational trauma becomes a metaphorical black hole that endlessly absorbs the energy of the therapist and the client, filling their life only with hopelessness. Having "positum" vision makes it possible for the therapist to avoid getting caught in the drama triangle when working with trauma, which is a very common effect (Remmers, A., Boessmann, U.). For example, in my practice, I saw that the expectation of a Russian attack in the autumn-winter of 2023 actualized transgenerational traumas of the II World War for many of my clients. And those of them with whom we were able to work through this experience (react to it and rethink it in the balance of "to know" and "to love" basic capacities) turned out to be much more prepared for the real invasion in February 2024 than the other people around them. And this "readiness" helped them to save not only their lives and health, but also help a large number of people around. Therefore, at a meeting with residents of a Polish city, it was important for me in my comments to share with them a "positum" vision of transgenerational trauma and prompt them to further think in this direction. It is a positive interpretation that allows the client to find his own unique meaning of the situation, and this is what moves him from the position of an object to the position of a subject of his own life.

Katarzyna Marquardt (psychotherapist, Trainer, Director of Positive Psychotherapy Center in Katowice, President of the Polish Association of Positive and Transcultural Psychotherapy, Poland).

"*To begin discussing transgenerational trauma, it is necessary to pause and engage in a deep reflection process that analyzes repetitive patterns concerning many dimensions (collectively, considering the history of past generations, as well as individual stories touching the roots and meanings of decisions made). A moment is needed to stop the defense mechanisms related to denial and negation.*

The residents of the town, by raising the topic of silence surrounding death, simultaneously opened many paths leading to understanding themselves, individual motives, choices, and actions, as well as the impact of past experiences on the future, not only of individuals but of the entire community. Opening this process allows for the creation of a collective pattern model and, in the future, the building of corrective experiences. In the cycle of the initiated therapeutic process, it would be particularly valuable and meaningful on both levels (collective and individual) to support the process of "understanding themselves with love, care, and precision" (R. Neborsky, J. Have-de Labije, 2017), which can help transform experiences and build positive perception of previous experiences of guilt and shame, so they are read and accepted as "a compass that serves constant dealing with oneself and others with love, honesty, and respect" (R. R. Neborsky, J. Have-de Labije, 2017) to past experiences, their own experiences, thus opening up to the future and rebuilding the much-needed, yet severely disrupted, bonds. Because what was harmed in the relationship can only be healed within it."

Christina Winkler- (psychotherapist, supervisor, Vice President of FF2P in charge of the Europe/Research Commission; Chair of the Chamber of Individual Members at the European Association of Psychotherapy, France):

"*This first four-hour meeting, the first of its kind, gave the opportunity to learn from each other: for the psychotherapists, learning was about listening to the stories and how people related to the current problem of addiction and drug abuse in order to get a better understanding of how trauma through the local story is underlying personal and family life; for the present witnesses, it seemed important to be listened to, but also to hear some professional positions about addiction, trauma, and even transgenerational transmission. Addiction and drug abuse concern individuals and families, and possibly everyone in the city can be impacted. Therefore, this issue becomes a field of exploration for a better understanding of how social interaction is at least partly driven by former traumatic experiences. In order to understand these intricate connections, research needs to and can be done with the help of the people deeply touched through their personal difficulties of living a healthy and valuable life.*"

Summary

The consultation in the town revealed the deep roots of social and addiction problems that trace back to the town's history. This process showed that historical and social conditions influencing current issues cannot be ignored. To effect true change, the community needs to openly discuss its problems, understand its history, and work together for the future. The actions taken in the town were the first step in a long process of healing and building a healthier future for the community. The described town became the first in post-war Poland to address historical collective trauma related to the Holocaust, the elimination of communities with psychiatric diagnoses and the Polish intelligentsia, as well as further repression during Communism, due to the intensification of symptoms affecting young people in the next generation. Considering there are hundreds of towns with similar histories in Poland, it becomes a symbolic "patient zero."

Key Points

1. **Impact of Transgenerational and Historical Collective Trauma**: This chapter examines the theoretical aspects of transgenerational trauma and historical collective trauma, highlighting their long-term emotional and psychological effects on subsequent generations. It focuses on the mechanisms of trauma transmission and their impact on contemporary mental and social health.
2. **Clinical Case of a Small Town in Poland**: As an illustration of the theoretical discussions, the chapter describes a clinical case involving the residents of a small town in Poland, who are experiencing collective re-traumatization due to the historical impacts of Communism, Stalinism, and World War II, using positive and transcultural psychotherapy.
3. **Importance of Resilience Factors**: The chapter emphasizes four areas of the balance model in positive and transcultural psychotherapy, which include relationships, achievement, imagination, and body. Resilience factors are defined as the resources and strategies that enable individuals to effectively cope with difficulties and potentially traumatic events.
4. **Five Stages of Consultation as Therapeutic Tools**: The consultation in the town, structured with the approach and five-step strategy of positive transcultural therapy revealed the deep dynamics of conflict and symptoms by identifying patterns and areas requiring special attention, such as dissociation, denial, or negation.
5. **Recommendations for Addressing Trauma**: The chapter recommends open discussion of issues, community engagement, working with grief and restoring hope, as well as education and historical awareness as key elements in overcoming transgenerational and historical collective trauma. It also highlights the need for external collaboration and openness to change.

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Positive Psychotherapy in Promoting Resilience and Hardiness

Liudmyla Serdiuk and Olga Lytvynenko

Introduction

Uncertainty and unpredictability have become today's realities. This requires a person to have resistance against negative and traumatising influences, to be capable of overcoming everyday difficulties, adapt to changing conditions, interact effectively with other people and be successful in various spheres. The Covid-19 pandemic has caused a serious health crisis around the world. Undoubtedly, the most serious challenges are wars, natural disasters and unexpected and unpredictable circumstances that cause significant and massive traumatisation of the population [1]. In addition, situations may periodically appear in the life of each person that become a test for his/her resilience (for example, stress because of bullying, pressure at work, unfavourable or complicated relationships, etc.).

The concern of mental health professionals regarding the negative consequences of stressful and traumatic influences such as burnout, depression, anxiety, and PTSD has caused an unprecedented interest in the phenomena of personal resilience, since understanding the clinical utility of a client's positive traits, such as resilience, justifies a balanced perspective in psychotherapy, in which clinicians work on promoting distress [2].

Modern studies show that resilience is important for effectively overcoming stressful and traumatic events: according to the theory of post-traumatic growth [3], personal *resilience* plays a key in relations between a trauma, PTSD and post-traumatic growth; [1] have shown that resilience and psychological distress correlates inversely. In this regard, the mechanisms developing human capacity for positive psychological adaptation in adverse life circumstances should be studied because of mental health specialists' significant practical interest in effective psychotherapeutic interventions supporting personal resilience, that is, his/her ability to cope independently with negative influences. Therefore, the problem of finding ways to preserve and strengthen a person's mental health and effective means promoting his/her psychological well-being is a promising direction of research in modern psychology and psychotherapy.

Studies on the consequences of Covid-19 and other traumatic effects evidence that, first of all, internal (personal) resources can help resist negative psychological effects and contribute to the preservation of a person's psychological health. In particular, numerous studies show that a sense of hope, self-efficacy, the presence of goals and meaning in life, developed cognitive-affective

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_52

personality spheres, resilience, optimism, wisdom, spirituality, a capability to use effective coping strategies to overcome stress are positive factors underlying positive mental health, cushioning adverse effects [4–6].

An example of external personal resource is, of course, social support, which can contribute to adaptive outcomes [7] and is a reliable predictor of posttraumatic growth (Linley & Joseph, 2004). At the same time, [8] has shown that stress and worsened well-being occur when a person is sensitive to the perception of external stimuli and does not have enough resources to transform negative environmental impacts or change them.

If we consider an impact of traumatic events on a person from the standpoint of positive psychotherapy, holistically, in the concept of "positum" [9], we can state that each person, thanks to his/her biographical experience, develops personal sensitivity and vulnerability to emotional traumas, which under unfavourable circumstances can lead to emotional and psychological disorders. At the same time, a person has unconscious spiritual resources and is able to actively participate in the healing process. In other words, as [10] notes, every person is able to integrate their experience of a disease into personal development and thereby strengthen their mental toughness.

The purpose of this section is to conceptualise the phenomenon of personal resilience, operationalise its complex holistic structure and show the possibilities of Positive Psychotherapy methods and tools for examining and development of internal resources supporting personal resilience.

Since resilience and other related phenomena, in particular hardiness, stress resistance, coping strategies, are considered as a psychological analogue of a person's life core, i.e., a person's individual ability to move on his/her own path to mental health resources [11], then a systematic consideration of resilience as signs of healthy and socially acceptable personality functioning also substantiates the possibility of conceptualising the mental health model.

Resilience, understood as mental toughness, describes the processes and mechanisms of a person's positive adaptation in the face of an adverse situation and after overcoming it [10]. Thus, resilience determines the protective factors necessary for a person experiencing stressful situations that threaten his/her internal and external integrity.

Literature Overview

In the last two decades, the phenomenon of resilience has become an extremely popular topic in various fields where difficult and extreme life situations are researched. The term "resilience" is quite widely used; it appeared for the first time in the works of E. Werner and her colleagues R. Smith, who in 1955 began a foundational study of the phenomenon of resilience. Despite such a long history, resilience has gained popularity as a psychological phenomenon only in the last two decades [12].

"Resilience" (from the Latin re – back, salire – to jump) literally means "to jump back". In psychology, resilience means a person's capacity for continuous normal, healthy functioning after exposure to adverse or threatening conditions or situations [13]. Most often, personal resilience is considered as an ability to maintain one's full functioning during and after such influences and as the equivalent of efforts made for self-improvement and for overcoming adverse circumstances in one's life [14, 15].

The existing numerous definitions and interpretations of resilience have many common features, in particular, that it is a *process and result* of successful adaptation to difficult life events or "challenges" [16]. However, despite the significant interest in studying and attempts to conceptualise the phenomenon of resilience in the mental health sciences [2], there is still inconsistency in its definition; so, there are different concepts of resilience [13]. This complicates the further development and practical implementation of a *resilience development model and, accordingly, intervention strategies*.

First, let us examine the terminological differences between the concepts of "resilience" and "resiliency". The difference is that resilience is a

process, and resiliency is a person's property (or ability). Resiliency research was focused on the study of protective factors that contribute to personal resilience. Resilience measurement scales (RS), which are widely used in empirical research on resilience, are based on this approach [17]. We should note that this approach was characteristic, to a greater extent, for an initial stage of resilience studies [18].

There is now a broad consensus that resilience is an adaptation process that goes beyond the construction of personality traits [18]. But, a central requirement for determining resilience in both process-based and outcome-based approaches is exposure to significant adverse events. From this point of view, resilience can be determined only if a person has been exposed to risk. If a person has not been exposed to risk or traumatic impact, it is not possible to assess his/her procedural stability. In our view, resiliency, as an ability, allows a person to assess the degree of risk and organise his/her activities in adverse conditions. Thus, according to the theory of posttraumatic growth [3], the stability of personality traits and resiliency as a trait, and not as a process, play a key role in relations between trauma, PTSD and posttraumatic growth [19].

Therefore, although modern definitions of resilience emphasise the process and result of adaptation, the approach based on personality traits determines the integrity of the phenomenon of resilience and is an important part of it, as it characterises resilience as an ability not only to recover, but also to develop stability resources after negative impacts.

For clarity, we will present these three components of the definition of resilience in Table 52.1:

So, taking into account the complex nature of resilience, as well as many biological, psychological, social and cultural factors that interact to determine how a person reacts to stressful and traumatic situations [23], we can say that resilience occupies an important place in the continuum uniting stressful and traumatic influences, illness, positive functioning and personal growth.

This continuum suggests that resilience exists to various degrees in different areas of life, for example, a person who adapts well to stress at work may not adapt well in his/her personal life or in relationships [24]; resilience manifests itself in different degrees, that is, resilience is not the opposite of psychopathology or PTSD, but a resource that prevents psychopathology, for example, trauma survivors having PTSD can be just as resilient as trauma survivors without PTSD [21].

Table 52.1 The definitions of resilience

Resilience as a property (or capacity)	Resilience as an adaptive process	Resilience as a result of experience
The ability to bend without breaking, to recover and perhaps even grow despite adverse life circumstances [20]	A dynamic process that leads to positive adaptation in the face of significant adversity [13, 21]	A conscious effort to move forward using a meaningful integrated positive way as a result of experiences gained from adverse circumstances [22]

Thus, generalising existing approaches [3, 20–22, 25–27], we can define resilience as a dynamic mechanism that uses the system of human resources to adapt, preserve health and maintain well-being under the influence of adverse factors. This system of resources helps a person withstand adverse factors, assess and forecast the degree of risk, adapt, recover, and reorganise for successful functioning, survival and even development, by integrating the gained experience.

Thus, resilience manifests itself in at least three ways, which include: *recovery, resistance, and reconfiguration* [28].

Because it is difficult to operationalise resilience as a complex multidimensional dynamic phenomenon, its comprehensive examination is also significantly difficult, as well as construction of a methodology for the development of resources supporting resistance against negative influences.

The concept of "hardiness", introduced in the work of S. Kobasa [29], as "stress hardiness", which is quite often used in the literature as a synonym for resilience, is very close in meaning. In reality, these are not synonymous phenomena,

as hardiness is part of a more complex resilience construct.

In modern studies on difficult and extreme life situations that are based on such concepts as strategies for overcoming difficult life situations, post-traumatic stress disorder or correlations between stressful life situations and diseases, hardiness is considered as a psychological analogue of a human life core, as a stress buffer. This is, in a way, a measure of a person's overcoming and adaptation to given circumstances, as well as a measure of efforts made for self-improvement and for overcoming adverse circumstances of his/her life [14, 30, 31].

Hardiness is a stable multidimensional personal property conceptualised as a set of cognitive and emotional-behavioural characteristics. It is a set of attitudes and beliefs about oneself and the world that provide the courage and motivation to turn challenging situations into opportunities for growth, as well as the ability to remain healthy and remain highly efficient despite strong stress [14, 29, 30].

Therefore, a person's hardiness is a system of stable qualities, understood as a person's internal resources under of his/her control, precisely because he/she can change and rethink what contributes to the maintenance of his physical, psychological, and social health; it is an attitude that gives life value and meaning in all situations [15].

As an integral personal characteristic, hardiness covers the phenomenology of formation of a person's basic life attitudes, adaptive potential and behavioural strategies for overcoming stressful situations and existential anxiety; hardiness manifests itself in overcoming difficulties, as the ability to keep faith in oneself, to be confident in oneself, one's capabilities, as the ability for effective mental self-regulation.

The content of hardiness is conceptualised as a three-component construct [15]: commitment, control and challenge.

1. Commitment means a person's attitude toward him/herself, the surrounding world and the nature of interaction with it that allows a person to feel significant and valuable enough to be fully involved in solving life tasks, without paying attention to existing stressful factors and changes.
2. Control means control over circumstances, searches for ways to influence the results of stressful changes, as opposed to falling into helplessness and passivity.
3. Challenge helps a person to be open to the surrounding world, other people, society. The essence of this openness lies in perceiving life events and problems as a challenge and test for oneself personally.

The concept of hardiness [32] is very close to the concept of salutogenesis proposed by A. Antonovsky [33]. The key concept of salutogenesis is the sense of coherence, which is understood by A. Antonovsky as a special set of attitudes (disposition) that determine a person's ability to be aware of cause-and-effect relations in life events and be confident in one's own abilities and resources, self-help skills that ensure control of one's own life. Despite the considerable proximity of the phenomena of coherence and hardiness, the mechanisms of their regulation are significantly different: A. Antonovsky emphasises the role of emotions, while S. Maddi attaches more importance to cognitive processes.

Positive Psychotherapy (PPT) combines these phenomena. From the point of view of Positive Psychotherapy, a person has two fundamental interrelated capacities: the capacity to love (primary) and the capacity to know and understand things (secondary). Both sets of personality capacities (primary and secondary) form a matrix of capacities, where primary capacities mainly regulate the emotional aspects of personality, while secondary capacities mainly determine a person's behaviour and actions [34]. In turn, resilience and hardiness can be interpreted as key concepts of salutogenesis, that is, a person's ability to move along his own way to the resources of mental health, the health of society in the context of culture [11]. Therefore, resilience and hardiness are considered as signs of healthy and socially acceptable functioning.

Conceptualisation of Resilience in Positive and Transcultural Psychotherapy

The searches for an in-depth answer about how to help a person develop his/her internal resources contributing to psychological resilience and post-traumatic growth have led us to the understanding that methods used in PPT (techniques, tools, rules) allow creation a comprehensive buffer of resources both in the inner world of a client and in his/her environment. These resources help him/her to cope with traumatic experiences and are the factors contributing to post-traumatic growth. For example, via the PPT technique of positive reinterpretation, a client learns to form a new perception of a situation. Thus, positive reinterpretation helps reduce stress associated with this situation, gives a broader perspective, turning a crisis into an opportunity. And when a client obtains an opportunity to find positive or developmental aspects of the experienced crisis, to see a new opportunity in it, his/her participation in the therapeutic process and the chances for recovery increase significantly.

By its nature, Positive Psychotherapy focuses on development of a person's inner potential, creating an accepting therapeutic context, an atmosphere of safety and trust, where a client can work on his/her own effective problem-solving strategies, which ultimately increases his self-efficacy in stress management [35]. Such positive transformation under very difficult life events is the experience of positive changes occurring during dealing with extremely difficult life crises [3]. Based on this, Positive Psychotherapy can be called an *example of salutogenic psychotherapy*, focused on the development of health resources, since the psychotherapy goal is to stimulate a client's natural healing mechanisms, that is, his/her internal resources, aimed at finding life balance and mental health, or harmonious functioning in four spheres of life: physical, cognitive, relational and spiritual, i.e., their proportional development and adequate consumption of energy in these spheres of life.

Salutogenic psychotherapy is not just the flip side of a pathogenetically oriented view of health. This is a fundamentally new model of psychotherapy [35, 36]. Salutogenic thinking challenges classical psychotherapy, prompting us to discuss aspects of resource activation, reconsider our view of a person, define modern concepts of health and illness, understand the context of a client's life and environment, evaluate treatment strategies and goals of psychotherapy.

The idea of A. Antonovsky [37] about the origin of health is influenced by systemic theoretical considerations: health is not a normal, passive state of equilibrium, but a labile, active and dynamically regulated event, since a person's feeling healthy or ill significantly depends on his/her individual, psychological attitude to the world and his/her life. Being a humanistic model of therapy with a psychodynamic understanding of the conflicting and behavioural therapeutic elements, Positive Psychotherapy is very close to the salutogenic concept of Antonovsky [33, 37] as it defines the factors necessary for effective salutogenic work [38]:

- A positive, humanistic image of a person that leads to confidence in one's individual capacities.
- A positive interpretation of diseases, which leads to therapy orientation from diagnosing symptoms and deficits to development of resources.
- Holistic approach. Through such concepts as the Balance Model, a person is considered comprehensively in body, mind, emotions and soul, and all these elements are taken into account in therapy. Thus, the Balance Model is a model of health and explains what health is and how it looks in practical life (salutogenetic thinking).

One of the psychotherapy goals in PPT is to help a client understand and mobilise his/her own resources in order to bring the four areas into dynamic balance. In this context, it is important to note the influence of a positive image of humanity that has the concept of a healthy person and indicates the path to health [35].

The Balance Model is based on a holistic human image used in Positive Psychotherapy. In this context, Peseschkian [35] talks about *biological-physical, rational-intellectual, social-emotional and spiritual-soul spheres and* capacities of a person. Although the potential of all four spheres is inherent in each person, the environment and socialisation emphasise some and ignore others. Each person has his/her own preferences regarding how to resolve appearing conflicts. Which forms of conflict resolution are preferable by a person largely depends on the gained experience, especially in childhood.

Therefore, the systemic characteristics describing the phenomena of resilience and hardiness, as well as the mechanisms transforming actions of stressogenic factors selected through the prism of life-sustaining beliefs (S. Maddi) allow PPT to describe the four spheres of the Balance Model and the four spheres of human life functioning. The types and structure of resilience identified in numerous studies [39, 40] show the holistic nature of this phenomenon and, thus, conceptualise the content of the four spheres included into the Balance Model:

- *The Body/Senses.* The ability to cope with changes and recover from physical exertion, illness and injury; the ability to self-understand, positive self-acceptance, to show attention, compassion and benevolent attitude toward oneself; development of a consistent, integrated identity.
- *The Achievements/Activity*. The ability to adapt to changes; cognitive flexibility; acceptance of a real situation and search for adequate new solutions; belief in oneself; the ability to learn and develop, act and overcome difficulties; the ability to perceive oneself as a survivor and the conviction that even in difficult situations one can continue to act until the goals are achieved; changing existing thinking patterns and ways of evaluating experience as a change in strategies from a "victim" to a "survivor".
- *The Contacts/Relationships*. The ability to receive and provide social support to others; to create and use a network of personal contacts; to regulate emotions, to be aware of them and to be in touch with one's inner world; the transformation of destructive emotions into a choice regarding the interpretation of a difficult life situation; the ability to reach for and receive understanding support and regain important social roles.
- *The Future/Meaning*. Having a meaningful goal; the ability to look to the future with hope; to take responsibility for one's life with the belief that difficulties and crises are temporary; searches for new goals and meanings.

By determining the four spheres of the Balance Model, we can build a model for the development of resilience resources, at least those that are under the control of a person him/herself.

The Positive Psychotherapy framework says that a healthy person does not mean a person without problems, but the one who has learned to cope adequately with emerging conflicts can eliminate problems. That is, according to the Balance Model, *a person who evenly distributes his/her energy in all four spheres is healthy.*

The Positive Psychotherapy tools identify deeply the *internal resources of a person's resilience*. In particular, the Wiesbaden Inventory for Positive Psychotherapy and Family Therapy (WIPPF) has a great examining potential; it asks for self-assessment of characterological and personal qualities (secondary actual capacities in relation to one's own behaviour and primary actual capacities in relation to oneself), which are manifested in the psychodynamics of conflict reactions that underlie a person's worldview concepts (WIPPF) [41].

Psychological studies using WIPPF obtained a significant empirical pull of data that determines the factors and predictors of a person's resilience and hardiness. The primary actual capacities of particular importance for the development of personal resilience and hardiness are: hope, time, faith/meaning, patience, contact, love/acceptance, and confidence in ones' own capacities. Resilience is also facilitated by secondary capacities such as punctuality, sincerity, reliability, diligence, justice [42–46]. Both primary and secondary capacities

can either contribute to increasing resilience or undermine it, depending on their content, development and direction [45].

Development of Resilience with the Tools of Positive and Transcultural Psychotherapy

In Positive Psychotherapy psychological stability or resilience is considered primarily as a person's ability to adequately differentiate and flexibly manage their potential, as well as actual abilities in accordance with internal needs and situational context [10]. Above, we presented the conceptualisation of the phenomenon of resilience through the Balance Model, emphasising the importance of both primary and secondary actual capacities for personality stability and viability.

Primary capacities, such as hope, faith/meaning, patience, contact, love/acceptance and confidence in one's own capacities, are fundamental elements contributing to a person's resilience. These capacities developed in a therapeutic relationship allow a client to form a reliable internal support. For example, the capacity to love and acceptance is formed from childhood through contacts between a baby and his/her mother and can be developed during positive therapy thanks to a therapist's capacity to show love and acceptance. Arno Remmers notes: "The emotionally meaningful, active and physically-interactive therapeutic relation can here have a healing quality, that means how we are, how our attitude towards the client is of a higher impact than what we "do" or which method we apply" [47]. This emphasises that a therapist's warm and accepting attitude promotes the internalisation by a client of this ability both at the level of bodily experiences and at the level of interpersonal relationships. Moreover, emotional support expressed through empathy, care, acceptance, intimacy, encouragement or participation creates a therapeutic context supporting resilience development. A. Remmers continues: "Emotional support is the offering of empathy, concern, affection, love, trust, acceptance, intimacy, encouragement, or caring. It is the warmth and nurturance provided by sources of social support" [47].

The development of resilience is also strongly supported by secondary capacities, such as punctuality, sincerity, reliability, diligence and fairness. These abilities contribute to flexibility and adaptability, helping a client to respond adequately to life changes and challenges. In the process of Positive Psychotherapy, a therapist helps a client to understand and develop these capacities, giving him/her an opportunity to consider their application in various areas of the Balance Model, such as the body, the achievements/activity, the contacts and the future/meaning. As Sari and Eryilmaz say: "By experiencing post-traumatic growth, individuals enter a new, more adaptive balance in the dimensions of body, achievement, relationships, and spirituality" [48]. Thus, the post-traumatic growth actually becomes a process of life balance restoring in these four spheres.

The Balance Model used in PPT includes the following four main areas:

1. *The Body/Senses:* In this area, such primary capacities as sexuality-tenderness, love-acceptance and trust develop. These capacities are formed at the early stages of life, through bodily experiences and contact with a mother. As A. Remmers notes, understanding one's own bodily sensations and feelings is an important step toward understanding and interpreting one's own reactions and relationships: "To differentiate the feelings and body reactions in the next step we can translate them into the underlying contents of capacities, values, conflict contents, and relationship patterns in relation to the patient's experience and history" [47]. This means that the ability to provide emotional support and physical contact plays an important role in therapy, contributing to resilience at the level of the body.
2. *The Achievements/Activity*: In this area, therapy is aimed at helping a client to discover new opportunities and develop skills that allow him/her to adapt to new life circumstances and challenges. As noted in the study

organised by Sari and Eryilmaz [48], the psychological growth after trauma becomes a process of recovery and adaptation in the area of personal achievements and goals: "The area of work and achievements—the discovery of new possibilities" [48]. A client learns to see new perspectives, develop his/her commitment and find internal resources to achieve his/her goals.

3. *The Contacts/Relationships:* Improving and deepening relationships with other people contributes to a sense of social support and security, which is an important component of resilience. Actual capacities within this sphere are: openness to others, the ability to empathise, empathy and sincerity in communication. As the Eastern folk wisdom says: "Shared joy is double joy; Shared sorrow is half a sorrow". Therapy helps a client realise the importance of meaningful relationships and develop the necessary social skills.
4. *The Future/Meaning*: This sphere includes spiritual changes and rethinking the meaning of life. In the therapeutic process, the emphasis lies on finding meaning in life's difficulties and using spiritual resources to strengthen internal stability. As Rumi says: "Where there is ruin, there is hope for a treasure". Revising life values and goals helps a client find new sources of inspiration and motivation, which in turn strengthens his/her resilience and ability to cope with life's difficulties.

Within the framework of Positive and Transcultural Psychotherapy, Patricia Badecka [42] investigated how to transform a traumatic experience into post-traumatic growth, relying on the achievements of modern neuroscience. She emphasises that the traumatic experience contains not only elements of suffering, but also the potential for personal growth. This approach requires a more in-depth analysis of the factors that contribute to this phenomenon and allows a person to use therapy as a means of turning traumatic consequences into an opportunity for positive changes.

The main goal of therapy, as Badecka emphasises, is to activate a client's natural self-healing mechanisms. A whole arsenal of tools is used during therapeutic work: from metaphors and parables to humour and transcultural elements that help a client look at his/her problems from a new point of view. This, in turn, contributes to transiting the perception of oneself from the position of a "victim" to the position of a "survivor", which is an important step toward restoring internal balance and harmony:

1. Transculturality: One of the key approaches that Badecka describes is an appeal to universal values and principles borrowed from various cultural traditions. Using a transcultural approach, a therapist creates an atmosphere of acceptance, safety and trust, stimulating a client's self-help and self-healing mechanisms. Badecka notes that this approach helps a client find internal resources to overcome trauma and strengthen psychological stability. "Trauma brings with it both suffering and the possibility of growth," she writes, pointing to the importance of integrating diverse cultural practices and values in order to create a richer and multi-level therapeutic process [42].
2. Proverbs and humour: An important PPT tool is used parables, folk wisdom and humour. This approach helps a client to look at his/her problems from a different, new perspective. As Badecka notes, the use of metaphors and parables opens up new points of view for a client, shows possible solutions and gives the opportunity to choose [42]. For example, the parable of a palm tree that, despite the heavy stone, was able to grow deeper to the soil to reach a water vein and become the tallest tree in the desert, shows a client that difficulties and obstacles can become a source of strength and growth.
3. Positive reinterpretation: The technique of positive reinterpretation teaches a client to regain control over his/her perception of a situation. This helps to reduce stress levels and promotes hope and humour, which are important for rethinking and accepting experiences. As Badecka writes: "Positive reinterpretation helps reduce the level of stress

associated with a given situation, gives a broader perspective, inspires hope and humour, gives the opportunity to learn, make sense, draw conclusions—turns a crisis into an opportunity" [42]. When a client can find positive or developmental aspects of a crisis, his/her involvement in therapy and the chances for healing increase significantly.

A telling example of how PPT techniques are applied is the story of Anna, a mother who lost her son because of leukaemia [42]. During the treatment of her son in the hospice, Anna actively used psychological support, and after his death, experiencing grief, she began therapy. At the end of the therapeutic work, at the stage of expansion of goals, discussing the sphere of meaning and fantasies, Anna decided to become a volunteer in the hospice.

Anna noted: "Since I started volunteering in our hospice, helping both parents and children struggling with this terrible disease, I have noticed that my life has begun to change. Please, do not misunderstand me, this does not mean that I have accepted the loss of my son. It will probably never happen. But I feel that his life and death have acquired a second meaning. I know that through my experience I can give a lot to those who need it. I know what they are going through, I was there myself, I know what it is like. It may sound strange, but for the first time since the day of his death, I feel needed, satisfied and happy" [42].

This example demonstrates how a client can find new meanings and goals, despite the experienced tragedies, in the process of positive psychotherapy. Anna preserved the memory of her son, but her life began to change, which indicates the post-traumatic growth through the acquisition of a new meaning and active participation in social activities.

Thus, Positive and Transcultural Psychotherapy provides powerful tools for developing resilience, helping patients not only to cope with the consequences of traumatic events, but also to achieve personal growth and harmonious functioning in various spheres of life.

Summary

Stresses, traumas and crises are a part of human life, and sooner or later each of us will face them. This will lead to long-term stress for someone, but someone will come out of this trial stronger, coping with challenges and returning to a meaningful life [23]. It is impossible and inexpedient to avoid traumatic events, but it is possible to develop resilience and internal resources, which are a buffer protecting against negative psychogenic factors.

Positive and Transcultural Psychotherapy by N. Peseschkian is a modern method of psychotherapy that successfully integrates both traditional and modern methods and techniques of working with clients. Being, on the one hand, a psychodynamic method, PPT focuses on building deep therapeutic relations, when a client's primary abilities can be developed in a warm and supportive environment. These opportunities presuppose the development of fundamentals for resilience and hardiness. On the other hand, the humanistic view on a person and resource orientation of PPT allows integration of different aspects of resilience as a phenomenon: an ability, a process and a result. Simultaneously, secondary abilities are formed through learning in therapeutic relationships within the proposed psychodynamic and humanistic approach that help a person build new coping strategies, be adaptive and readaptive in the modern turbulent world.

Traditional PPT techniques such as a transcultural approach, positive reinterpretation, parables, metaphors, humour and stories, used in combination discoveries in neuroscience, strengthen our hope for effective psychotherapy and the development of resilience with the help of PPT.

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Facilitating Immigrant Adaptation Through Positive Psychotherapy

53

Polina R. Efremova

One of the valuable ideas in positive psychotherapy (PPT after Peseschkian) is clarity of terms, concepts, and ideas. Nossrat Peseschkian, who himself experienced life in different cultures [1], deeply believed in the importance of helping a client to master the skill of self-help, so that the person could not only learn how to solve new life challenges but even become "a psychotherapist" for his own family. Following this principle, this chapter, like many other texts in PPT, is meant to be useful not only for mental health professionals but also for all types of readers who would seek instruments of self-help to cope with immigration.

"The term immigration refers specifically to international migration that is relatively permanent in nature. Immigrants are those individuals who have moved to a new country on a relatively permanent basis" [2]. Refugees are a specific type of immigrants who fled their motherland due to dangerous situations (war, persecution, natural disasters, etc.), but as they are explicitly described in another chapter, this chapter will be dedicated to immigrants in general, both voluntary and involuntary.

Immigration leads to intergroup contact of two or more cultures, which is believed to influence both the immigrants and the host-group [2, 3]. That is why it is crucial for the well-being of all groups to understand how to support the process of acculturation of immigrants. There is plenty of research dedicated to the major group attitude toward immigrants (for example, [4]) and to formation of attitude toward minority groups in society (for example, [5–8]), but this chapter will be built according to the resources a psychotherapist has while working with immigrants. Therefore, we will only briefly touch on the social psychology of groups, to the extent where it will be needed for a more profound understanding of psychotherapeutic processes with immigrants.

Problems Society Meets While Integrating the Immigrants

One of the common beliefs among natives is the idea of "zero-sum", which means that whatever goods immigrants gain, they are taken from non-immigrants. This idea provokes negative attitudes toward both successful and wealthy immigrants and immigrants who do not do well economically [2, 25]. Other threats perceived by the country of origin's inhabitants are a threat to health (immigrants as transmitters of new diseases) and to safety (immigrants as possible terrorists). There could also be a perceived value-related threat, when natives believe that letting immigrants save their traditional values may weaken the host coun-

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_53

try's values. What is remarkable is that the zero-sum concept is more common among people who demonstrate a higher social dominance orientation (believing in hierarchy and inequality). The less democratic the views of the host-country population are, the worse will be the attitude toward the immigrants [2]. The relation of the native population to the ethnic diversion is also important. Green et al. [27] mention two categories of definitions of nationhood: ethnic and civic. Ethnic definition requires common ancestors, culture, and traditions, which makes it impermeable for the immigrants, who can never become "true" members, while civic definition is based on learning the local language or acquiring citizenship, which lets the immigrants to be accepted as full members of the national in-group.

Psychological Problems That Immigrants Face

Immigration is one of the life events that changes an overwhelming number of factors in the life of a person. These changes touch everything, starting from essential physical habits (different food, different weather to adapt to), continuing with social and financial status (which often lowers after immigration), total change of social environment (social isolation, new contacts, leaving behind friends and family) and work conditions, finishing with significant changes in the area of meaning (different traditions, different values, different background, often uncertainty about the future).

The long list of changes and losses, coupled with confusion regarding the rules of interaction and the social norms causes what Berry calls acculturative stress. Acculturative stress is a response by people to life events that are rooted in intercultural contact. Frequently, acculturative stress includes heightened levels of depression (linked to the experience of cultural loss) and anxiety (related to uncertainty about how one should live in the new society) [3, 26].

Despite the stressors that an immigrant will inevitably face in any host country, one of the biggest possible stressors is connected with discrimination, prejudice, and social injustice [9, 15, 23].

Psychotherapists should also consider that the acculturation process is bidimensional, which means that immigrants not only absorb the new culture but also mourn the native culture and reevaluate traditions and concepts that they would like to save or give up. Even if the person did not run from a war or climate catastrophes and did not lose beloved ones, they still abandon their native culture, friends, in many instances language, places and habits, identity, which launches the same mourning process as the one connected with someone's death [10, 11, 28]. Eisenbruch states that massive social loss experienced by refugees through uprooting produces grief [10]. Thus, we could assume that immigrants do not undergo a linear process of adaptation but balance their relations with two cultures while trying to find their place in a new country. Even the motivation for immigration can be based on "push-pull" factors, when there could both exist the opportunities that attract people to the host-country and the reasons to flee the country of origin. The push factors could include poor economic situation, disasters, wars, etc., while pull factors could include education and labor opportunities, higher standards of living, etc. [28].

Adaptation to Immigration

During immigration a person should balance between finishing relations with one's homeland and adapting to the new culture both psychologically (as an inner process) and socioculturally (finding a job, restoring social status, integrating into the community).

Discussing adaptation, Berry [12] mentions 3 different dimensions:

- Psychological adaptation ("feeling well", maintaining high self-esteem, etc.)
- Sociocultural adaptation ("doing well", being successful at work/school, community)
- Intercultural adaptation ("relating well"—being able to build relationships across the cultural boundaries).

Considering the PPT balance model, one could say that sociocultural adaptation refers to the achievement dimension, intercultural—to the dimension of contacts, psychological—to the dimension of body and meaning/future. Regarding the latter, it could be helpful to differentiate two areas of personal (psychological) adaptation. Thus, one could discuss whether acculturation stress triggers psychosomatic diseases or the loss of meaning and reevaluation of personal values. Frankl [13], Antonovsky [14], and Prendes-Lintel [10] emphasized the importance of meaningfulness (the ability to find/create meaning of the hardships one is going through) as a factor of psychological resilience. At the same time, while being able to see meaning, immigrants can experience stress and exhaustion which will manifest in physical symptoms (Fig. 54.1).

One of the critical factors of adaptation is based on the choice of acculturation strategy.

"Acculturation is the process of cultural and psychological change that occurs as a result of contact between two or more cultural groups and their individual members" [3]. At the individual psychological level, it refers to changes in one's behavioral repertoires (changing food, dress habits, language, values, and identities) and one's adaptation to these intercultural encounters. An individual becomes a participant in a culture contact situation, being influenced directly by the external culture and by the changing culture of which the individual is a member. Depending on 2 axes (1) accepting-rejecting the host-country culture (motivated by the desire for contact) and (2) accepting-rejecting the native country culture (motivated by the desire to maintain native traditions)), four forms of acculturation strategy could be described: assimilation, integration, separation, and marginalization [12] (Fig. 54.2).

Integration is a strategy of combining inside the native culture with the host-culture. The person doesn't give up one's values and traditions but also absorbs the new ones, which helps build effective and respectful communication with people from the host culture, and become more successful in the achievement sphere. This strategy is believed to be the best for both sociocultural and psychological adaptation, which is supported by multiple research [23]. One of the hypotheses is that people with this strategy benefit and get support from both cultures.

Marginalization is a strategy of giving up the native culture, yet not absorbing the host-culture. It is believed to be the least effective strategy of adaptation and is connected with multiple risk factors.

Assimilation is a strategy, in which the person gives up his native traditions and habits to blend into a new society. Meanwhile, a big part of their inner world has to be abandoned.

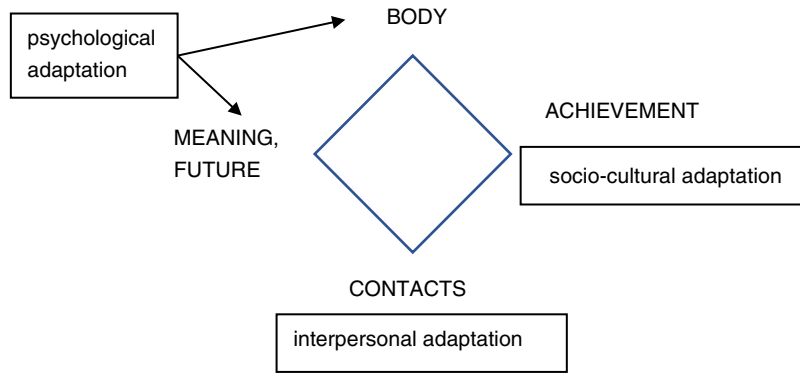


Fig. 54.1 Balance model and 3 dimensions of adaptation

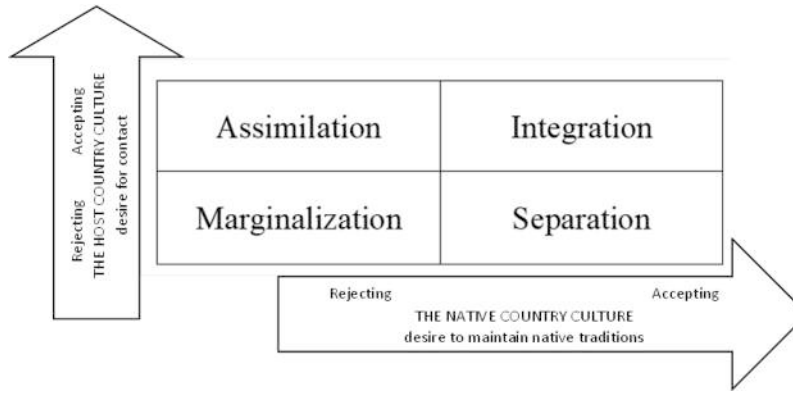


Fig. 54.2 Acculturation strategies

Finally, separation leads to isolation inside the minority culture, which continues to live according to the traditions of the homeland and resisting the changes that the host country can bring. The latter two strategies are believed to be mildly effective for adaptation.

At the same time, there are two nuances that can influence the choice of a strategy.

First, it is important to note that acculturation preferences involve both immigrants and members of the receiving society [2]. The way immigrants would prefer to fit into the new society is also influenced by which strategies this society allows or facilitates [4, 12]. Thus, integration is especially appropriate strategy in a multicultural society. Meanwhile, if the majority prefers assimilating the newcomers, assimilation would be the best in the working culture, while separation or integration could be used at home. Researchers agree that rejecting one of the two cultures (home or host culture) and attempting to live monoculturally has a negative impact on the immigrant [10, 23].

Second, the person can vary the strategies in different situations. For example, to succeed at work one can use an assimilation strategy while at the office, integration—in social life and separation inside the family [3, 12]. Talking in the language of positive psychotherapy, people who are able to unite concepts and traditions of their homeland with concepts and traditions of the host-county, while considering the expectations of host-society, adapt best of all.

Facilitating Adaptation and Integration into the New Culture Through Positive Psychotherapy

Any type of immigration, even voluntary one, which provides a lot of opportunities, confronts the person with a lot of losses. The often-used metaphor for immigration is “to uproot”. Forced immigration could be even more complicated, because one might have push motivation (to escape the country of origin) but have no plans for the future connected with the host-country, no fascination with the country, no wish to root there—nothing of what helps voluntary immigrants in the first months. It takes additional effort for refugees to start searching for opportunities that could attract them in the host-country. Nevertheless, Berry [3] cited research that emphasized that both strong push motivation and pull motivation (to move to a host-country) can cause difficulties. While the challenges of the strong push motivation were just explained, the strong pull motivation can create overly idealistic expectations, shattering when the immigrant meets reality, causing additional stress.

Therefore, the healthiest attitude is realistically optimistic. This is the attitude positive psychotherapy helps to form. The idea of *positum* describes the whole, not just the bad or the good part, which allows the person to react realistically to the present, and adjusting strategies here and now to reach an optimistic future.

The number of changes in an immigrant's life and attempts to reflect them all, as we wrote before, could become an additional source of stress. In fact, every single area of life is affected by immigration, from physical needs to existential questions, the meaning of life, plans, relations, and careers. It could be important to change the level of perception or switch the focus in the situation to make this experience bearable and potentially enriching, increase resilience, and mitigate the impact of acculturative stress. Several instruments from positive psychotherapy could do that:

1. "zoom-out" from details to the whole picture, where we could observe the pattern of investment of one's time and energy in different areas of life; it is possible with an instrument from positive psychotherapy (PPT) called "balance model".
2. "zoom-in" from places, habits and relations to the emotional needs behind them; PPT suggests a theory of primary capacities and associated emotional needs and can be used to understand one's cravings in each situation.
3. "zoom-out" from behaviors of people in the host-country to the concepts and broader ideas about behavior and relations considered "normal" in this particular culture; can be formulated in terms of "actual capacities", suggested by Nossrat Peseschkian.
4. "zoom-in" from the idea of national attitude to immigrants in the host-country to the experience of positive personal interactions (interpersonal communication proved to lower prejudice about the outgroup both for majority and minority [15]). In PPT it can be described by changing focus in the model of relations from the WE dimension to the YOU dimension.

The following instruments are described so that they can be used during therapeutic sessions, as home tasks, or as self-help instruments.

Balance Model in Immigration

The balance model is an instrument in positive psychotherapy that allows one to approach one's life holistically. It provides an opportunity to look not only at the particularities that make up life, but also at the overall distribution of resources across four areas. These areas are traditionally named (1) body, (2) achievement, (3) contacts/relations, and (4) future/meaning/fantasies.

The body includes sensation and perception, nutrition, sleep, tenderness, sexuality, relaxation, sport, appearance, and dressing.

Achievement is usually associated with any intentional activity, profession, work, education, house chores, career, and money handling.

Contacts/relations include partnership, family, friendship, acquaintances, strangers, and social engagement.

Meaning/future/fantasy/imagination "operates by multimodal representation of reality, including semantic symbols, which allow the thinking per se, assumption of meaning/causality, forecasting of the future, perception of mental space, and self-regulation" ([16], *verbal discussion*). This dimension actively uses means of religion, meditation, reflections, beliefs, ideas and values, imagination, and fantasies [17].

One of the basic exercises with the balance model is the distribution of time and energy across four areas of life. It allows us to assess the actual value of each area and associated resource distribution, thus switching focus from the detailed list of losses to a bigger picture. During the exercise one reflects how much one is ready to invest in each area to feel satisfied. It could be both a diagnostic tool, allowing to observe the significant changes in all spheres of the client's life, and a tool for intervention, allowing the patient to build plans despite the situation of uncertainty and a lack of usual resources.

Basic instruction suggests evaluating, how much time and energy the person invests in each sphere, how satisfied he/she is with the actual distribution, and what is the ideal future distribution for them, what prevents them from reaching the ideal distribution and what could be done to reach it [16, 17].

Additional instruction for diagnostics of changes in immigrant's life:

1. Please mark how much time and energy you've spent on each area **before** the immigration. (after the client finishes). Please mark with another color how much time and energy you are spending on each area **after** the immigration (Fig. 54.3).

For example, the person tended to spend 20% of his energy for the body, 40% for achievement, 30% for contacts, and 10% for fantasies before the immigration. After the involuntary immigration the amount of energy invested in activity remained the same (though performance at work could decrease), the huge amount of energy is spent on plans, fears and fantasies, there is scarcely enough energy left even for the basic contacts with the family, while body needs are considered "unimportant" and thus abandoned (also for a reason of difficulty to fulfill the needs in usual physical needs in the new environment).

Instead of focusing on painful nuances: "They don't sell my favorite cottage cheese here," or "I couldn't find a suitable cosmetologist," or "My favorite capoeira group stayed in my home country"—we can offer the client several new criteria for adaptation.

5. Compare the distribution of time and energy globally in four areas before and after immigration.
6. Evaluate satisfaction with the current distribution.
7. Experiment with filling the balance model according to different criteria:
   - Leave the percentage ratio as it was before departure, but fill each area with new content (instead of capoeira—surfing, instead of a cosmetologist—cheap and high-quality massages).
   - Talk about the need for attention for each area and consciously redistribute efforts ("now my body needs much more care," "building new connections abroad requires more energy from me than at home, therefore I should not push myself to socialize as actively as in my home country").

In this way, we shift the client's focus from what is not available in the new environment to a higher, meta-level, where they can reflect on how much attention and effort they want to devote to each area to feel comfortable.

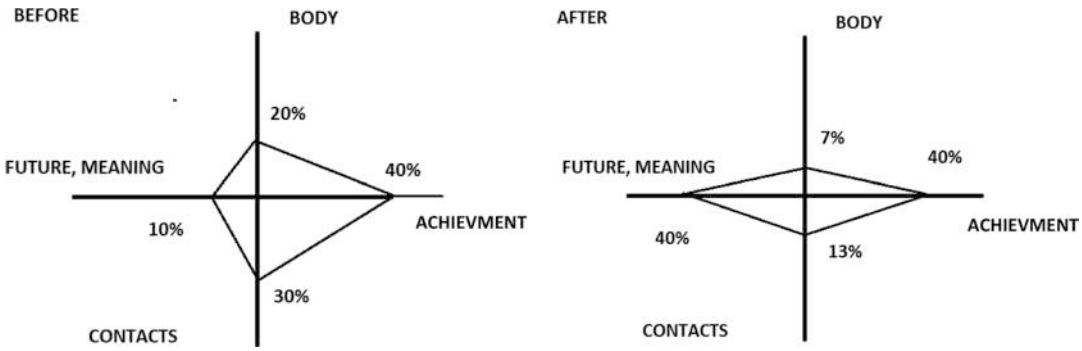


Fig. 54.3 Example of energy distribution through the balance model before and after the immigration

What Do We Have in Common and How Are We Different?

When arriving in a new country, a person inevitably discovers a lot of differences with the host-culture. Moreover, Berry [3] addresses studies which show that the more different the cultures are, the less positive the process of adaptation is. This can give rise to both internal alienation (they are different, I don't want to build relationships with them) and fear of external rejection (they will expose me, they will understand that I am not like them, and they will not accept me). The other extreme is when all differences are denied, and a person turns to the locals, assuming complete mutual understanding, which can lead to a number of awkward social situations.

N. Peseschkian [18] suggested meeting new cultures and even new people inside one culture with two questions:

1. What do we have in common?
2. How are we different?

Both questions are critical to adaptation. Similarities and differences can be identified using actual capacities and the balance model. One can effectively analyze similarities and differences identified in actual (primary and secondary) capacities, allowing one to pinpoint behavioral norms, inner needs, and processes of emotional attachment. According to Nossrat Peseschkian, every human is born with two basic capacities: to love (affectional-motivational sphere) and to know (cognitive-behavioral sphere), which differentiate into actual capacities and grow or get suppressed under the circumstances in one's primary socialization environment. Primary capacities develop from the capacity "to love" and could be described as emotional needs that we have and can fulfill for the partner. Secondary capacities develop from the capacity "to know" and could be described as social norms [17]. The indicative list of actual capacities is in the table below (Table 54.1).

Following questions that might help to identify similarities and differences:

Table 54.1 Actual capacities

Secondary capacities	Primary capacities
Punctuality	Love
Cleanliness	Modeling/ideal
Orderliness	Patience
Obedience	Time
Courtesy	Contact
Honesty/candor	Sexuality
Faithfulness	Trust
Justice	Confidence
Diligence/achievement	Hope
Thrift	Faith/religion
Reliability	Doubt
Precision	Unity

Is the hosting society more primary or secondary-oriented than the one of my origin?

What of the relevant capacities that are important to me are shared here?

What capacities are given more importance than I am used to? Which of them are given less importance? How does this make me feel? Why? How do I think this society came to have this particular attitude toward these capabilities?

For example, if we consider the author as a Slavic immigrant in two countries—Turkey or Portugal—we could collect some subjective impressions of the differences in capacities in these respective countries.

First of all, Turkey and Portugal are more primary-oriented countries than Russia. We could also assume that while in the balance model of Russians achievement and relationships take the major roles, in Turkey it would be relationships and meaning, while Portugal seems to focus more on body and relationships. Regarding differences in secondary capacities, punctuality seems to be much more important for Russians, than for Portuguese or Turkish, while politeness is much more important for Portuguese than for Russians. Love and Patience, especially toward children, are much more common for Portuguese and Turkish cultures. Responsibility seems to be on one level in Russia and Portugal but lower in Turkey. Cleanliness in Russia and Portugal seem to have the same value. Faith seems also to be alike in intensity.

Moreover, some of Peseschkian's [19] observations about Western and Eastern cultures and their attitudes to the capacities match with these observations. Knowing more about the norms of the cultures helps people lower prejudice, which often bases on misunderstanding of others' behavior. These subjective reflections can as well help the person move from generalized shock reactions to differentiated ideas of what is alike and what is different, which can help new immigrants adapt and understand the behavior of others better.

From Things, Actions, Places, and People to Feelings and Needs

Moving to a new country deprives a person of so many familiar objects, activities, hobbies, and contacts that returning everything "as it was" is almost impossible. Moreover, attempts to make it "just like home" can cause frustration when some elements repeatedly fall out of place and do not match expectations. Obsession with specific ways of satisfying needs can lead to a feeling of confusion, hopelessness, and despair. Therefore, one of the most important tasks when working with people who have emigrated is to teach them to notice the needs and feelings behind each element they miss. What feelings did I have walking in a favorite park? Meeting what needs were giving me these feelings? The following two exercises could help clients "zoom in", refocusing from the lost objects to the primary needs associated with them.

Ex. 1. "The feeling of home".

While living in the same place, we get used to connecting our feelings and needs to some stable places and activities. Therefore, in immigration we often struggle with the sense of "irreplaceability": "They don't have my favorite park, my beloved cafe, my sports club here", etc. It is hard but necessary to switch focus from physical details that were present in the home-country to the feelings it gave us, emotional needs it covered. The ability to separate a physical object from the feelings we crave to feel is an essential capacity, which needs to be developed to cope with the amount of changes an immigrant faces. For example, it is crucial to understand that the sport club was not really about sport and staying healthy, but more about brotherhood, having a circle of people connected with some shared activity and regular leisure (primary capacities: unity, contact). Therefore, some random gym will not fulfill these needs, while a touristic club, a quiz team, or a music band could replenish these emotional needs. Please, remember what you miss in your home country. Now read the list of primary capacities and try to notice which one was met by this particular object. Come up with three available ways of fulfilling this need.

Ex. 2. "Indestructible roots".

Peseschkian worked extensively with the metaphors, often taking patients' comparison as a starting point. Returning to the metaphor of the roots, we could transform it to help the clients adjust by searching for the roots connected to the values more than to the place. For example, "My roots are in respect of the private life and identity of other people, and I will not stay in the country which tries to break in humans' private lives". Sometimes it helps to formulate immigration as a choice made to save the deepest, the most important roots (values, beliefs).

Choosing Acculturation Strategies According to the Stages of Interaction with Homeland and Host Country

Berry himself writes that the strategies could change over time. Meanwhile, in positive psychotherapy we assume that any relations (including the one with the country) go through three stages that develop sequentially: fusion/attachment (being dependent, non-critical, identifying with the country), differentiation (active learning and asking questions, comparison of declared values and reality, entering confrontation to protect one's individuality), detachment (loosening

of connection, integration of absorbed concepts and personally learned experiences, independence). The progression through the stages cyclically leads to a new spiral turn: detachment is followed by attachment yet at the next level of interaction. Thus, relations deepen over time and through the crises (Peseschkian 1987).

How might these three stages be applied for adaptation in a new country:

At the **fusion (attachment) stage**, one may feel enchanted by the new country, experience the desire to be like its inhabitants in everything, and have an obsessive desire to meet all the cultural norms of this country (sometimes such people are said to be "more German/Canadian/Italian than natives"). Also at this stage, cultural traditions and the native language may be sacrificed (for example, even at home the family speaks the language of the new country in order to learn it faster), and critical attitude toward the rules of life in the new place is reduced.

This happens due to the fact that a new place of residence, as a rule, provides opportunities/safety that were not available in the home country, allowing one to satisfy needs that have been deprived for a long time. This creates a strong motivation to accept any "terms of the deal" in order to finally feel how a significant deficit is being replenished.

At the **differentiation stage**, we normally begin to actively build boundaries and reclaim our space. The needs that we were ready to push aside at the beginning are becoming more noticeable and unfulfilled, and we are no longer satisfied with only what our current place of residence offers us. We notice which needs were satisfied in the old place of residence and we are not ready to simply give them up for the sake of new ones. During this period, the level of dissatisfaction with the new country often increases, many rules and customs cause irritation, and an emphasized adherence to old traditions and habits may appear.

At the **separation (detachment)** stage, we become able to recognize the possibilities and limitations of the new country, while understanding how most of the processes work and, therefore, being able to navigate the new environment and satisfy our needs without confronting it. We feel more independent.

But, since the adaptation is bidimensional, the stage of relations with the homeland should also be considered. For example, a person forced to leave their country may still be too attached to it and thus feel alienated or even hostile to everything different in another culture and imposed rules. They will fiercely defend their "native traditions" and way of life, hope for a chance to return, and lack the motivation to fit in.

The stage of differentiation from the native country will encourage a person to nihilistically reject all the values of the homeland, to emphasize separating himself from his compatriots.

The stage of detachment from the country of origin may look like the ability to feel independent, yet internalize what remains truly valuable to us from our culture. The person can see what the home country has given them and objectively perceive the reasons for not coming back.

While clearly understanding the complexity and multitude of different factors, influencing the strategy of acculturation that one can choose, we would still like to demonstrate some correspondence of the four strategies of acculturation, presented by Berry [12], and the stages of interaction with 2 counties.

The fusion stage with the host country, especially supported by the differentiation stage with the country of origin, can probably lead to an assimilation strategy. A desperate desire to be accepted, to fit in, forces a person to reject everything "old", "from a past life" and absorb the traditions of a new environment. This method, it would seem, is quite adaptive, but it has a significant side effect. One can never become as part of the new culture as someone who was born in it. Such excessive imitation can provoke a feeling of falsehood and even rejection in local residents. Etching out one whole's prehistory can deprive him or her of depth and flatten their experience and personality.

Another strategy of acculturation—separation—could be provoked by the differentiation phase with the host-country. Dissatisfaction with what is "wrong" in the new place of residence can encourage immigrants to withdraw into com-

munities based on national/religious characteristics. Reflecting on the bimodality of the adaptation process, one can also assume that separation can be associated with the stage of attachment to one's native country (for example, due to forced immigration).

In turn, the tendency to differentiate from both the country of birth and the host country (akin to adolescence's withdrawal into nihilism) can provoke marginalization as a way of acculturation.

Integration as a strategy of acculturation demands some degree of detachment. In order to preserve the values of one's own culture, absorb the new values of the host country and reconcile them with each other, on need to recognize oneself not only as part of a particular culture, but as a separate independent person who is capable of making choices, changing one's views, and preserving within oneself the most resonant ideas of the outside world.

Nowadays traveling has become faster and cheaper, which allows people to visit their home country regularly in peaceful times. Nevertheless, in case of permanent immigration the relationship with the country-of-origin changes. Life goes on, both for the person and for the home-culture, the friends and family members left behind. And therefore, it is essential to reach the stage of detachment in these relations. It is not about forgetting, but more about introjecting the essential values and traditions, recognizing and accepting the consequences of one's choice to immigrate (loss of many contacts, loss of context, change of cultural background).

Techniques That Can Help the Client Move Forward from Stage to Stage

"Gratitude to the homeland"—the client plays as a conversation to the empty chair or writes as a letter all the gratitude to his home-country (for the good memories, for building the character, for the beloved safe places, etc.). Then it is important to acknowledge that this stage of life is over ("Thank you for everything"/ "I will keep it in my heart", etc.). It will help to unfreeze the mourning if the person is stuck in denial, while distinguishing the exact values and memories helps to degeneralize the feeling of loss and move closer to the differentiation stage.

"Accepting the reasons for departure"—the client plays a conversation with the empty chair or writes as a letter the objective reasons for leaving the country. The client recognizes the lack of safety/opportunities/justice, and learns to verbalize it without guilt or shame (sometimes through multiple discussion with the therapist or another accepting partner).

"Integration"—writing down the national qualities, celebrations, traditions, and beliefs that are dear to the client and which they want to maintain. Marking those valuable traits of character which were formed not only by the family but by the culture in which they grew up. Reading them aloud, embracing them as a part, which is not outside now but inside. Depending on how the client feels, some form of final conclusion could be said aloud ("My home is inside", "I am all these things and much more than them", etc.).

"5 Stages to Connect to the New Country"

The client is suggested to go through the 5 classic stages in PPT:

1. "Observation"—the therapist suggests the client take the role of investigator, eagerly collecting knowledge about the new place, traditions, weather, people, food, music, etc. The client is motivated to mark the most exciting findings, pay them attention, and enjoy them as much as possible.
2. "Inventory"—here the client is suggested to analyze what is similar and what is different in their country of origin and the host-country.
3. "Situational encouragement"—the client is motivated to emphasize all their successes in adjustment to the new environment, and notice their capacities that help them cope.
4. "Verbalization"—the client clearly verbalizes what works in their adaptation strategies and

what needs to be done/changed to help the client adapt.

5. "Broadening of goals"—building a step-by-step plan based on the conclusions from the previous step. As this step is oriented to the future, there is an additional helpful idea the therapist can implement. The most uncertain part of the immigrant's future is the next 2–5 years, which often makes people frustrated, as middle-length goals are the ones we are used to making. The therapist reframes future plans into the discussion of the nearest future (step-by-step plan) and the long-term future ("what I would like to see in my life in 10-15-20 years"), which does not depend that much on the hardships and challenges the client is going through at the moment.

Model Dimension (Model of Relationships)

Another tool of positive psychotherapy is the model of relations. This rhombic model represents four spheres of relationships that we can build in our lives:

I—relationship with oneself.
You—relationship with a partner.
We—relationship with people, groups, society.
Primal We—relationship with the world as a whole, something bigger (for example, faith) (Fig. 54.4)

Each of these areas undergoes significant pressure during the immigration process. Despite the fact that fundamental changes occur in the "We" and, partly, "Primary-We" areas, these dramatic life changes also exacerbate conflicts related to the attitude toward oneself ("I") and partnerships ("You").

Thus, if one's attitude toward self is rather negative and critical, then disorientation in new living conditions, temporary isolation and difficulties in finding a new environment can increase the feeling of one's stupidity, inadequacy and worthlessness.

In couple relationships, isolation in the process of adaptation to a new environment and increased chronic stress, exacerbate pre-existing discontent in the couple and elevate the level of mutual demands. Also, a resource deficit can simply discard the couple to the automatic patterns of reactions that grow from the basic concepts, which could be excessively rigid or even harmful for the relationships. It can hit the couple by surprise, if they were succeeding to overcome the harmful family patterns before the immigration. Numerous studies were done during COVID-19 pandemic, exploring the influence of isolation and stress on couple relations: Lee S.J.

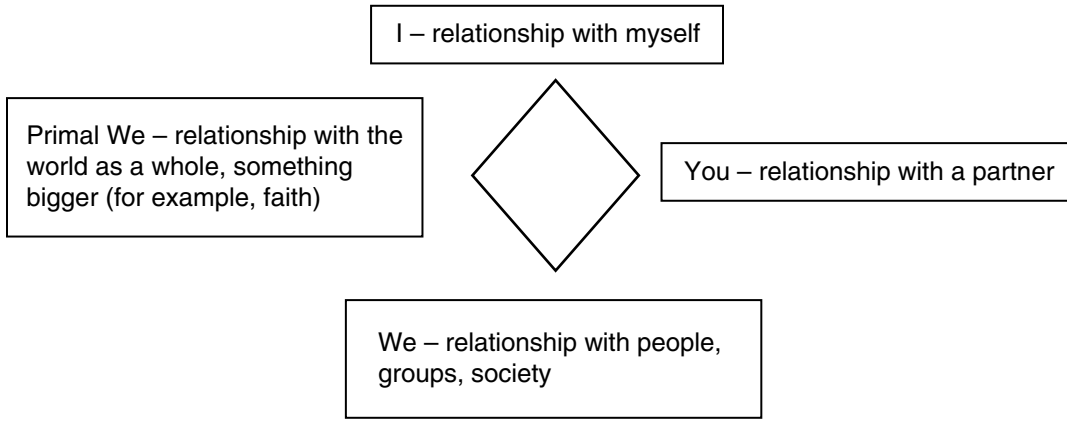


Fig. 54.4 Model of relations (model dimension)

et al [20], Wisyaningrum et al. [21] state that these conditions caused worsening relationship conflict, even among couples who start with high levels of dyadic coping.

The Primal We area undergoes significant changes due to the separation from the roots—the lifestyle that parents and previous generations led. Adapting to a new country also requires a revision of some values, which can create a serious crisis of meaning. The picture of the world as a whole changes significantly and expands, filled with new ideas that may have been less accessible in the country of origin.

Of course, the greatest changes occur in the "We" area: the environment changes, the rules of social interaction change, the group's attitude toward me and mine to the group can play a key role in a personal adaptation. Since childhood, we learn to be open to others or isolated, to strictly differentiate between "us" and "them" or to accept people with their differences. In a familiar environment we can afford to be reserved and even hostile to many groups, while feeling included in society and the general context. On the contrary, in immigration our social survival depends on our openness and accessibility to develop contacts. As we did not have a chance to absorb the rules of functioning of this society in childhood as natives did, we find ourselves dependent on contact with others to learn the rules and principles of interaction, get hints about what "goes without saying" in this group. At the same time, our ability to present our differences in a respectful and gentle form is also essential, as the inhabitants of the host-country will as well need to adapt to our cultural features.

Therefore, a therapist should keep in mind that although the first help to the immigrant will be connected to stress-release, help in navigation through cultural differences and psychoeducation, some of the stress factors will have psychodynamic roots and, therefore, will require psychotherapeutic interventions after the acute stress is diminished [24]. On the other hand, processes that might look like they have roots in childhood could reflect the acculturation process, like cases of child-parent relations or marital conflict, presented by Kohatsu et al. [22].

Conclusion

Positive psychotherapy (positive and transcultural psychotherapy) has a rich inventory to work with the problems connected to adaptation and acculturation after immigration. The cultural sensitivity of the method encourages the therapist to keep attention on individual norms, the norms of his cultural background and the norms of the current environment, which allows to decrease the tension one can feel due to the noticeable differences of lifestyles between their country of origin and host-country. The concepts of the balance model and actual capacities can help change the client's focus from specific objects and behaviors to the needs, emotions, and traditions behind them.

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54 Positive Psychotherapy Approach for Patients with Personality Disorders

Gabriela Hum, Anca Claudia Bragaru, Sergiu Maxim, and Diana Pop

Introduction

Personality refers to the dynamic integration of the totality of a person's subjective experience and behavior patterns, including both conscious and unconscious cognitions, habitual desires, behavior patterns, views, and fears [1]. From the perspective of Positive Psychotherapy (PPT after Peseschkian), personality is a unique combination of actual—primary and secondary—capacities and how the person exhibits these capacities in her/his thoughts, feelings, behaviors, and attitudes. The development of this combination (personality) is influenced by the biological characteristics of each person, the environment and culture in which someone grew up, and the education the person received. "Personality develops step by step, over time, along with the person's capacities. Each stage in their development has its own problems and conflicts, or developmental crises. If they are resolved harmoniously, they are assimilated into the stages of identity, and the personality can flourish in harmony with the person's development." [2]. Well-developed primary capacities and a flexible development of secondary capacities are the background of a healthy personality.

Based on its humanistic aspect, PPT places the patient at the center of evaluations and interventions, including those with personality problems. Thus, Peseschkian rarely wrote about interventions specific to a personality disorder (PD). He was a pioneer of the approach 'one size does not fit all,' often mentioned by researchers and therapists studying the efficiency and effectiveness of psychotherapy, such as Norcross [3], who stated "only matching psychotherapy to a disorder is incomplete and not always effective". Peseschkian approached the person and its unique development, the over- or under-developed capacities that, in relation to self or others, lead to difficulties. He focused his interventions on the resources the patient has and his/her uniqueness.

Personality structure refers to the internal organization of traits and components of personality. It is the basic framework, or internal architecture, that supports and shapes the overall personality. It includes components such as fundamental personality traits (e.g., introversion vs. extraversion), defense mechanisms, core values, and patterns of thinking. It is a more specific and technical concept used in psychology to understand how the various traits of a person's

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_54

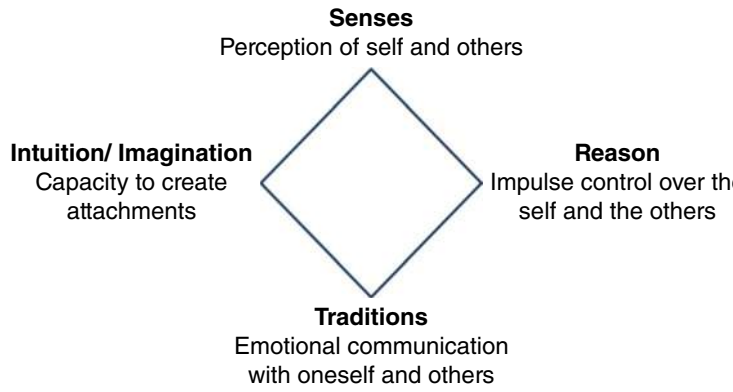


Fig. 54.1 Personality structure areas from Positive Psychotherapy (in bold) and Psychodynamic perspective

personality are interconnected and organized. Based on its psychodynamic influence, PPT describes the structure of personality as having four major areas (Fig. 54.1): the capacities of perception through the body's senses, the capacity to control impulses, the capacity for emotional communication, and the capacity to create attachments [4]. The comparison between the Balance Model and the psychodynamic model of personality (Fig. 54.1) is described in Chap. 8 of this book.

Peseschkian completed individual psychodynamics and psychogenesis by a model of sociodynamics and sociogenesis. In addition to the intrapsychic conditions for the development of a personality structure, that is individual psychodynamics, what shapes concepts and value models in sociogenesis and establishes a culturally shaped sociodynamics, are primarily social, cultural, and transcendental relationships [4]. The original family and the transgenerational transmission of values and concepts play an important role in personality and its structure development. The relationship between parents and children, the relationship between parents themselves, between parents and social environment, and their philosophy of life become models or counter-models for the person *(Model dimension)* and a way to develop relationships in his/her own life (with oneself and the others).

PPT has its perspective on personality development and a specific set of tools for addressing the uniqueness of patients with PDs.

Personality Structure and Personality Development from PPT Perspective

The basis of personality, primary capacities, implicitly developed in the early period of life, reflect the way parents and other caregivers have interacted with the child. Peseschkian mentioned that the way parents provide time, patience, and modeling to the child during the attachment stage becomes, through introjection, a way for the child, and then for the adult, to offer time, patience to oneself and others, and use the experienced model in stressful or frustrating situations [2]. The primary capacities developed later on—hope, acceptance, trust, tenderness, contact, and meaning in life—represent needs that every human being manifests in one form or another. A child surrounded by patience, with caregivers who adjust their time and pace to the child's rhythm and who convey congruent verbal and nonverbal messages, will grow up with a well-developed set of primary capacities and emotional balance. Lack of parental patience, limited or poor-quality time spent with the child, and inconsistent and incongruent messages can lead to the development of an insecure attachment and thus an unhealthy personality.

Secondary capacities also play an important role in the development of the personality through the way the child, and later the adult, adapts to the social environment and social norms, respects

and sets boundaries, and engages in age-specific activities and responsibilities. For a good balance in the development of the personality, it is recommended that from an early age (around 2 years old), the child begins to learn social rules such as punctuality, cleanliness, order, etc. In this way, love (in its forms described above as primary capacities) is accompanied by age-appropriate limits and responsibilities.

The unique combination of primary and secondary capacities developed to a certain level for each individual provides a picture of personality.

When talking about personality structure, PPT emphasizes the importance of self-esteem. Without mentioning it in his books, Peseschkian indirectly refers to this element of personality structure by specifying that one of the important steps in therapeutic intervention is situational encouragement or self-esteem enhancement. Self-esteem reflects an individual's subjective appraisal of themselves as intrinsically positive or negative to varying degrees.

Self-esteem can be described as having two important components: the intensity (level) of self-esteem and its stability (or variability) [5, 6].

- A person with high and overly stable self-esteem, often specific to patients with grandiose narcissistic PD [7–9], will rarely seek therapy. Often, we find the relatives of these patients in therapy, seeking help. These patients often grew up in environments where everything was allowed, there were no boundaries, and the child developed a sense of omnipotence.
- Low and unstable self-esteem, specific to patients with borderline, avoidant PD, and vulnerable narcissistic PD [10–15], requires actions with positive results and a stable and containing relationship from the therapist to stabilize self-esteem and help it to grow. These patients grow up in environments where parents exhibit ambivalent behavior towards the child, either through inconsistency between what they say and what they do or by alternating between behaviors of love and acceptance and behaviors of rejection and punishment.

A low intensity and/or stability of self-esteem will always induce defensive mechanisms that, at the unconscious level, will protect a person from unpleasant feelings and changes, and will block the understanding of unconscious material and access to the patient's resources. "The inner conflict has a paradoxical nature at the unconscious level, as it creates a kind of screen behind which the conflict 'ferments' until it 'overflows.' The function of this screen is to allow the person not to process the conflict" says Peseschkian, referring to the resistance role of defense mechanisms. These are creative compromise methods that a person develops unconsciously throughout life. In his books, Peseschkian mentions and describes several defense mechanisms such as regression, dissociation, identification, projection, rationalization, repression, humor, altruism, etc. [2, 16]. In PPT, significant emphasis is placed on the development of humor as a mature defense mechanism. Peseschkian used to say that "if we haven't laughed at least once with the patient during the therapy session, it means we haven't done therapy." Humor is like the salt in the soup of therapy, accelerating the insight and relativization of fixed concepts. In depth psychology, humor is understood as among the most mature defense mechanisms and overcomes resistance to changes in therapy, which were to be understood as the ability to hold on to the previous and to oppose changes [17].

On a more conscious level, adaptive coping mechanisms play an important role in the development and flexibility of personality structure. In situations of conflict or stress, energy distributed evenly across the four areas of the Balance Model can protect the person from the impact of stressors on the personality structure (Fig. 54.2).

Any structure is an arrangement of parts, interacting with each other under historically developed rules and thereby forming the functional whole. When there is a dynamic interplay between the elements of the personality structure, a "struggle" between what one MUST adhere to as norms (secondary capacities) and what the patient WANTS to do (primary capacities), self-esteem that balances this struggle, a

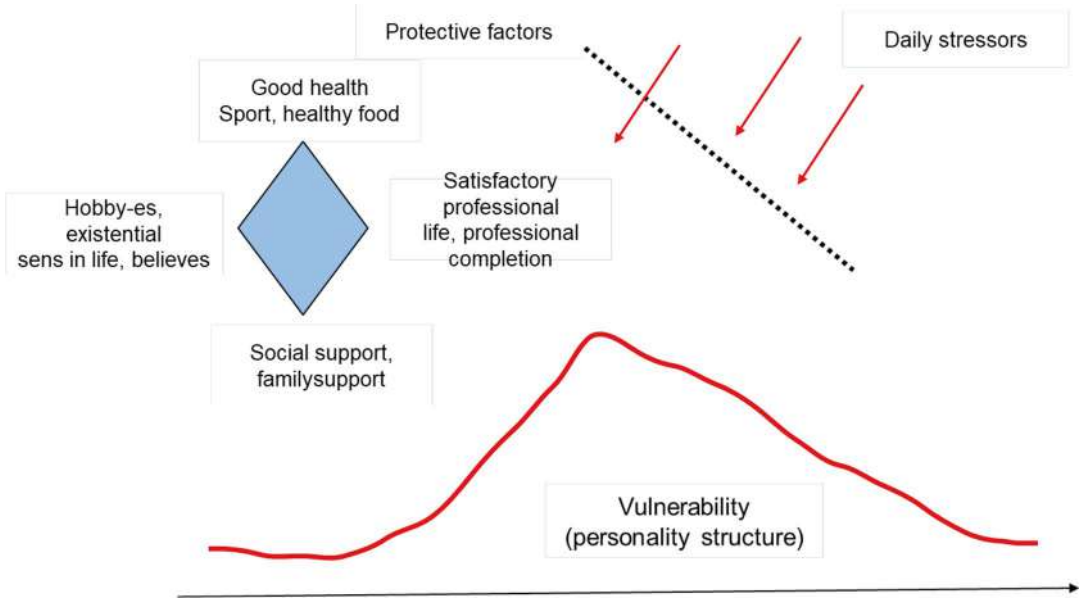


Fig. 54.2 Coping mechanisms as protectors of personality structure

filter provided by mature defense mechanisms, and adaptive coping strategies, we have a dynamic and harmonious integrated personality structure. PDs' or disintegrated personality structure arise when this dynamic is blocked by one or more elements of the personality structure, such as:

- Secondary capacities that are too rigid (over- or under-developed).
- Insufficiently developed primary capacities.
- Self-esteem difficulties (at the level of intensity and stability).
- Pathological or immature defense mechanisms.
- Maladaptive coping mechanisms.

A well-integrated structure allows the person in stressful situations to avail oneself of one's mental space and to regulate it via intrapsychic processes in such a way that she/he can also establish and maintain satisfactory relationships with self and with others. Disintegrated structural capacity is unavailable for usage because of its insufficient development or vulnerability under the pressure of internal or external stressors. [18].

Positive Psychotherapy Interventions for Patients with Personality Disorder

Dirty Nests

A pigeon kept changing her nest. The strong smell that developed in the nests over time was unbearable for her. She complained about this bitterly while talking to an experienced, old, and wise pigeon. The latter nodded a few times and said, "You change nothing by constantly changing your nest. The smell that bothers you doesn't come from the nests but from you."

In addressing patients with PDs, PPT uses several classical psychodynamic tools (therapeutic relationship, transference and countertransference processes, confrontation, interpretation) as well as a set of its instruments (e.g., the 5 stages of intervention, *positum*, 3 stages of interaction are only some of them). The goals for the therapy of patients with PD are related to how the patients get to function, socially and in relation to themselves, to value their uniqueness, and to explore, develop, and change their personality structure.

The intervention for patients with PD involves a five-step strategy, as described in previous

chapters: observation-distancing, inventory, situational encouragement, verbalization, and broadening the goals. Specifically, when working with patients with structural problems, the five steps may need to be repeated several times and can be bidirectional. Sometimes ruptures occur in the therapeutic relationship, requiring a return to the observation-distancing stage. Other times, the patient may react defensively during the verbalization stage, which requires returning to the situational encouragement stage. When the Broadening of the Goals stage is too short some patients (especially those with dependent PD) relapse into old patterns of functioning. Relational rupture and drop-offs are common in the case of these patients. Being a long-term therapy (over 50 sessions), during this process the patient may experience various life situations (losses, divorce, accidents, etc.) that interrupt the initial therapeutic process and require special attention. The therapist must always keep the structure of the intervention in mind to discern where they are in the process and what needs to be addressed or what boundaries need to be set to stabilize and develop the patient's personality structure.

Stage 1: Observation-Distancing

> A healthy person is not a person without problems, but a person who has the necessary capacities to solve them —N. Peseschkian

Sometimes this stage can take more sessions than in therapy for patients with intrapsychic conflicts (neurosis), and can overlap with the inventory stage. The focus of this stage is on understanding the patient's current situation, establishing the goal(s) of therapy and forming the therapeutic relationship through offering of positive interpretations (*positum*), paying close attention to the patient's first statement, and observing how they relate to the therapist from the very first session, as well as paying attention to the processes of transference and countertransference. In this stage, we start the process of reattachment. "What good therapists do with their patients is analogous to what successful parents do with their children" ([19], p. xi). Providing time, patience, and a model of relationship (with acceptance, hope, but also boundaries) from the very first therapy sessions can have a reattachment effect. Attention to the process of transference and countertransference (which will be described later) starting from this stage ensures material for subsequent interpretations and confrontations in the Verbalization stage.

An element that therapists need to consider during the initial session is the *first statement* of the patient. This sentence can often provide information about the patient's attachment style. Along with other information gathered during the next sessions, the therapist can determine what type of attachment the patient developed in childhood, which needs were not met, and what is the main area of intervention (the 5 A's model described below). Thus, we will have patients in the office who, at the first interaction with the therapist:

- Starts neutral discussions about the weather, the location of the office, politics, etc., unconsciously avoiding focusing on themselves and the issue that brought them to therapy.
- Starts discussions focused on the therapist and its role as a savior, idealizing the therapist.
- Or begin therapy with a focus on themselves and the reason they are in the office.

When a therapist encounters one of the first two situations at the initial meeting with the patient, it is important to leave a question mark in his/her notes regarding the patient's first statement. The therapeutic relationship is a microuniverse of the patient's relationships in their life. Therefore, the patient will repeat the same attachment style with the therapist, either clinging to the therapist and placing responsibility on the therapist's shoulders or avoiding real connection with the therapist. The elements gathered from the first therapy session, along with other aspects uncovered in the patient's life history, will help to more clearly and precisely formulate the patient's attachment style.

Positum for Patients with Personality Disorders

Peseschkian through the concept of *positum* (which means "factual," "given") used to say that

"diseases, disorders, and unsuccessful attempts to solve problems are not the only things that patients bring to therapy; they also bring the capacity to overcome them" [16]. As also Yalom wrote in his book *The Gift of Psychotherapy*: *What do patients recall when they look back, years later, on their experience in therapy? Not insight, not the therapist's interpretations. More often than not, they remember the positive supportive statements of their therapist* [20]. In this regard, using positums from the very first therapy sessions, in the Observation Distancing stage, provides an important foundation for building the therapeutic relationship and offers a reattachment space to the patient.

1. *Positum as the foundation of positive and negative reality.* In Positive Psychotherapy, this concept refers to the recognition that every aspect of a person's life, including negative traits and difficulties, is part of their reality and can be used in the therapeutic process [21].
2. *Positum as a positive reinterpretation of symptoms.* Positum involves reinterpreting symptoms or difficult situations in a way that highlights the individual's resources and capabilities. This transforms negatively perceived symptoms into opportunities for personal development [21].
3. *Positum as the recognition of latent capacities.* The term *positum* is used to emphasize that the symptoms and problems individuals face are manifestations of their latent capacities to react and adapt. In this sense, Positive Psychotherapy focuses on identifying and developing these latent capacities [22].

When we think about offering a positive interpretation, we highlight the capacities that help the patient cope with their current situation or the capacities that have been developed by that situation, or as Mentzos said the "function of dysfunctionality" [23]. For example, we see dependent attachment as the person's ability to rely on those around them (to take allies). As for avoidant attachment, we describe it as the patient's ability to be autonomous and rely on themselves, as well as the ability to test relationships before relying on others. The aggressive behavior of a patient with Borderline PD could be the capacity to be sincere in relation to others. Their emotional instability is their capacity to experience a wide range of emotions. Regarding patients with narcissistic structure, their ability to believe in themselves and to present themselves as they are is one of the essential capacities.

We will stop here by providing examples of positums. It is much more important for the therapist to understand the meaning of this technique and to practice formulating and understanding the effects of positive interpretation in their self-discovery process. When offering a positive interpretation of the symptom, the therapist must adapt the *positum* to the patient's life story and to what the patient reveals beyond their narrative. A 'by the book' positive interpretation may have effects contrary to expectations.

Therapeutic Relationship

Building a trusting relationship begins from the very first therapy session. The relationship between the patients and the therapist supplies the secure base that is the foundation for exploration, development, and change. The therapeutic relationship can provide a context for accessing experiences within the patient that have not—and perhaps cannot—be put into words (sometimes dissociated experiences). The relationship is also a context within which the patient, having made room for these experiences, can attempt to make sense of them. Accessing, verbalizing, and reflecting upon dissociated and unverbalized feelings, thoughts, and impulses help to shift in a more reflective direction the patient's stance toward experience [24].

In Positive Psychotherapy, the therapeutic process is described in terms of three stages of interaction or developing a relationship. In each session, as well as throughout the entire intervention process, the therapist implicitly sets a stage of attachment (the opening of the therapy session), a stage of differentiation (the actual intervention), and a stage of detachment (the closing of the session with feedback, conclusions, or homework). These stages become a functioning

model for the patient initially in therapy sessions and, for some, in daily life, especially in the case of patients with PDs. During the Observation Distancing stage, the therapist may notice the patient's need for development in how they build relationships. We may encounter patients who:

- find the attachment phase difficult as in the case of patients with avoidant PD,
- struggle with the differentiation phase—their ability to express their needs or what is happening with them honestly and politely is low, or they may end relationships where the partner/ therapist sees the world differently—as in the case of patients with borderline PD,
- or have difficulty with the detachment phase (they prolong session times or violate the weekly contact rule by requesting additional sessions or sending messages to the therapist between therapy sessions) as seen in patients with dependent or borderline PD.

Arno Remmers often describes the Observation-Distancing phase saying that in this stage, the therapist metaphorically places themselves behind the patient's chair and tries to view the world/ situation/ illness through the patient's eyes, without intervening, interpreting, or confronting the patient. The focus of this phase is on establishing the therapeutic relationship, setting therapy goals, and providing the first elements of reattachment: time, patience, and a relational model (with acceptance and boundaries). The main goals of therapist in the Observation Distancing stage are identification *(take your patient's view),* containing *(accept the patient's affects and emotions)*, acceptance *(accept sufferance),* helping the Ego *(sustain client decisions),* protection *(prevent harmful acts),* support *(help the client as mentor, coach, „parent").*

Stage 2: Inventory

> It is much more important to know what sort of a patient has a disease than what sort of disease a patient has—Sir William Osler

Understanding the patient's life history is the focus of this stage. In addition to gathering information about the current situation of the patient with PD, during the Inventory stage, we are particularly interested in the timeline of the patient's suffering and coping strategies. Knowing that the first signs of a PD typically appear in adolescence, we can specifically inventory this period. Earlier on, we are concerned with the patient's childhood, including the difficult situations they experienced and how they coped with them. Exploring the patient's life story, the therapist has the implicit objective of assessing the four areas of the personality structure: how the patient perceives themselves, others, and the world, how they control their inner and outer impulses (in connection with their body and sexuality, with money, technology, substances), how they emotionally communicate with themselves and with others, and how they establish attachments. A triple layer of assessment is always important when working with patients with PDs:

- The patient's story: what they say about themselves and others.
- The therapist's observation in the therapeutic relationship: how the patient's story is reflected in their interaction with the therapist and is consistent during the sessions.
- The therapist's observation of his/her reactions to the patient's story and reactions (countertransference process).

Transference and Countertransference Processes

Transference reflects experiences of earlier interactions that are unconsciously projected onto the therapist. At the core of the transference process lies one or more of the basic needs unsatisfied in childhood. These unsatisfied needs hold patients to what happens before and is not fully processed. They look for other relationships in which some of the original unsatisfied needs might get their chance to be fulfilled. David Richo [25] described the 5 basic needs (the model of 5A's) as being:

- Acceptance, affection—body and senses.
- Appreciation—achievement.
- Attention—contacts.
- Allowing/Autonomy—fantasy, spirituality.

We can connect these needs with the Balance Model and look at them as being the needs behind each transfer process that happens in relation to our patients. The following questions to the therapist are suitable for recognizing typical transference patterns [26]:

- Whom does the patient see in me, how/who will I be for my patient in therapy? (Role, family constellation, reference person).
- What does the patient see in me, as often in certain others? (Content, conflict issues, unsatisfied needs).
- What does the patient expect from me, as often from others? (unsatisfied needs).
- Which of my patient's reactions seem inappropriate, distorted, or incomprehensible to me in relation to the therapeutic situation?
- Does my patient talk about feelings, wishes, or views of third parties that could also apply to our therapeutic situation or to me as a therapist?

Countertransference includes all the sensations, actions, communication patterns, and associations that arise in the therapist on the occasion of a therapeutic encounter and are typical for this encounter. Remmers [26] described the types of countertransference as being:

1. Physical perception of feelings, posture, vegetative reactions.
2. Perceiving one's patterns of action, and therapeutic considerations as specific to this encounter.
3. Communication patterns and body language encounter.
4. Fantasies, images, desires, digressions, fears, memories of previous encounters or episodes of one's life.

There are significant relationships between patients' personality disorders and patterns of countertransference [27]. Therapist's tendency to criticism was associated with patients with paranoid and antisocial personality disorders. Patients with Borderline Personality Disorder (BPD) are prone to induce helplessness, overwhelming, and overinvolved responses. Patients with Narcissistic PD (NPD) and schizotypal PD induce disengaged therapist responses. Helpless countertransference was associated with patients with schizoid personality disorders. Avoidant personality disorders were associated with positive responses. Overinvolved therapist responses were associated with patients with Obsessive Compulsive PD. When working with patients with personality disorders the countertransference first is to be contained (Observation Distancing and Inventory stages) and then utilized to clarify and explore how the patient's internal object relations are being enacted in the therapeutic process (Verbalization stage). Countertransference reactions are important information about the basic needs of the patient [28, 29]. Therefore, therapists can make diagnostic and therapeutic use of their countertransference (Fig. 54.3)

Personality Disorders, Balance Model, and the 5A's Model

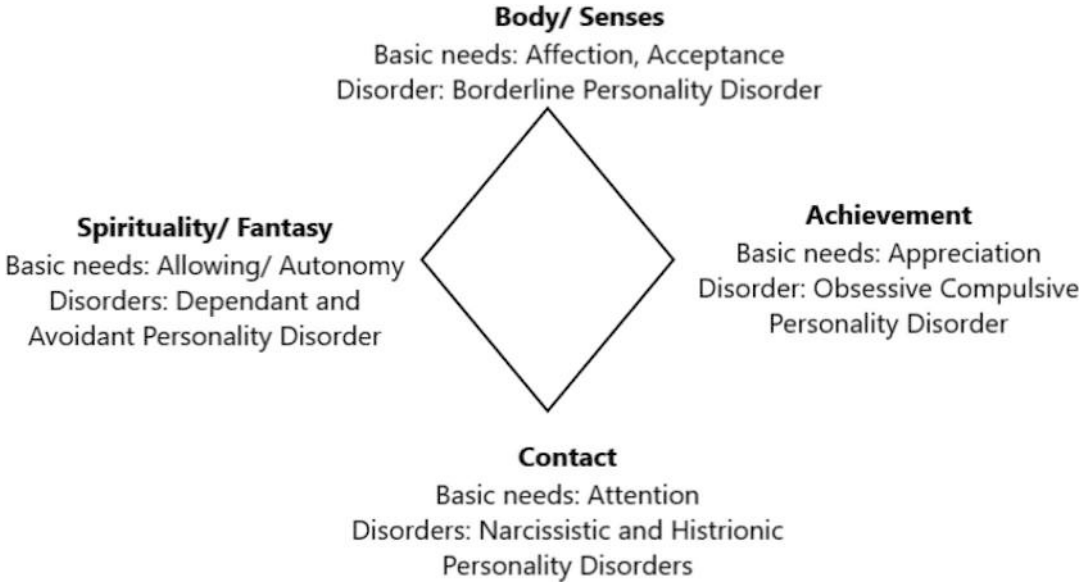


Fig. 54.3 Balance model, 5 basic needs, and PD

Borderline Personality Disorder (BPD) and Body Senses Area

It is said that "Our body is our house" (Corinthians, New Testament). For patients with BPD, this house is "lived in" by much aggression. The body, through self-harm behavior, compulsive eating episodes, unprotected sex that endangers the patient's life becomes the channel for expressing the impulses, anger and anxiety that they feel and are unable to control. Borderline patients' need for acceptance and affection is expressed and demanded by this accumulation of behaviors that signals to outsiders, the patient's internal chaos. H. Peseschkian [30] presents the case of a patient with BPD who experienced aggression and self-harm as methods of managing frustrations and negative feelings. Through therapy, these destructive behaviors were transformed into healthier communication skills and assertiveness. Instead of harming herself, the patient learned to express her feelings and needs constructively and to manage conflicts in a healthy way. Aggression, which is the patient's ability to be very open (sincere) and through this to ask for the acceptance and affection she needs, was channeled into a positive form of expression. The patient also managed to address past traumas, such as sexual abuse and her parents' divorce, through therapeutic discussions and exercises in self-reflection and mentalization. This helped her better understand and process the emotions associated with acceptance and affection [30].

Obsessive-Compulsive Personality Disorder (OCPD) and Achievement Area

Patients with OCPD have a tendency toward perfectionism, unconsciously trying to nurture the belief that they will receive appreciation and will balance self-esteem if they reach a certain introjected standard. They also have characteristics such as workaholism, undue preoccupation with productivity to the exclusion of pleasure and interpersonal relationships [31], which will cause one of the most affected areas to be the Work/ Achievement Area. Patients with OCPD are unconsciously guided by the belief that love and appreciation (primary capacities) can only be obtained through colossal efforts to achieve the

highest standards in life, in terms of orderliness, cleanliness, performance, achievement, justice, etc. [32]. Their lack of self-confidence is fueled by the lack of appreciation that they want to obtain through everything they do. From here a set of secondary capacities (orderliness, cleanliness, performance, justice, etc.) develops, which become the basic pillars of their functioning. These capacities shape the image of an inflexible and unspontaneous person but, at the same time, a responsible, steady patient, always punctual and up to date with the given tasks.

Narcissistic Personality Disorder (NPD) & Histrionic Personality Disorder (HPD) and Contact Area

He who loves himself will have no rival —Franklin

The approach of positive psychotherapy offers a valuable perspective on the resources that can help individuals with narcissistic personality disorder (NPD) function socially. In the context of the narcissistic structure, differentiating between healthy narcissism and narcissistic personality disorder (NPD) is crucial. Healthy narcissism involves high and stable self-esteem and resistance to external influences, whereas pathological narcissism is characterized by excessive dependence on others' opinions and maladaptive behaviors [33], low or high but unstable self-esteem. A high level of the capacity of time and the capacity of patience are two essential aspects and predictors of the narcissistic personality structure. For example, underutilization of time is a positive predictor of narcissistic traits, while a high level of patience, which involves the ability to delay immediate gratification, is a negative predictor [33]. In NPD pathology, the area of relationships is mainly affected due to their inability to offer time and patience, the two primary capacities that help a person connect to others. Patients with NPD are perceived as incapable of love and with low empathic abilities. They see people as objects to be used and then abandoned according to their needs, without any consideration for their feelings. Therefore, relationships are terminated shortly, usually when the other person starts to make demands or express their needs. These patients, in contrast, can love themselves, be loyal to their convictions, and take care of themselves.

On the other hand, the excessive need for attention and theatrical behavior characteristic of HPD patients highlights the Relationships area of the Balance Model in that there is an excessive need to be desired by others. In patients with NPD we are talking about a need for admiration from others, in patients with HPD we are talking about various symptoms that draw the attention of outsiders to some needs that the patient cannot ask for in another form. In specific PPT terms, patients with HPD can sense the sensitivities of those around them and connect to others through these sensitivities.

Dependent (DPD) and Avoidant Personality Disorder (AVPD) and Spirituality/Fantasy Area

In patients with DPD, there is an impairment in Spirituality/Fantasy, an area described by Peseschkian as beliefs about meaning, life, death, personal development, life goals, etc. The main characteristic of this disorder is that independence and autonomy have been associated with danger and anything that can bring independence into the patient's life, brings increased anxiety [32]. In this way, patients avoid making decisions or expressing their demands, clinging on to others, and unconsciously passing on the responsibility for decisions, wishes, and future. Their need for autonomy is inhibited and covered by the danger of the intense fear they feel to act and think independently. In terms of positive psychotherapy, patients with DPD can establish relationships easily and trust others easily.

When discussing patients with Avoidant Personality Disorder (AVPD), they experience the same difficulty related to autonomy, but in the sense of assuming too much autonomy. Repeated and unsuccessful attempts to attach to primary caregivers have led them to unconsciously perceive relationships as a source of anxiety and distress. The unpredictability of experienced relationships, essential self-doubt, and a mistrust of others, with the anticipation of humiliation or rejection, and the fear of aban-

donment [34], often cause these patients to "escape into fantasy" and live in a safe world created by themselves. The ability of these patients to create and use fantasy as a coping mechanism is one of their resources.

Stage 3: Situational Encouragement

The development of a reliable and realistically based sense of self-esteem is an important therapy objective when working with patients with PD. Situational encouragement is the stage in which the focus is on patients understanding and accepting themselves as they are, on maintaining reasonable standards by which to evaluate themselves, and on tolerating criticism and failure (or success, for that matter, the tolerance of which can also be difficult) without anxiety or loss of a sense of self-regard [35]. Although it seems like an easy stage with a lot of focus on resources and positive aspects, in the case of patients with PD this stage can bring relapses, drop-offs, or defenses that endanger the therapeutic relationship (either by the fact that the patient does not believe the therapist's encouragement, or that the patient becomes afraid of the relationship that starts to be too close) especially when patient perceive the therapist as being not enough authentic.

Situational encouragement involves the training of the key conflict (see Chap. 35 for details on the conflict model). Beyond the educational component, where the therapist describes the types of key conflicts and their characteristics, a discussion about the patient's key conflict helps with self-reflection and self-awareness. For patients who are still in denial about how they use politeness and honesty, feedback exercises from close individuals (family members, friends) can help provide a better reflection of this type of conflict. The patient and his/her surroundings play an active part in this stage in the understanding of the process of interaction and the uniqueness of the patient in relation to others. Once the key conflict is acknowledged, it can be trained and shaped into a more adaptive form (Peseschkian used to say in his courses that a person with a balanced key conflict can express their truth with a great deal of love). This training will later assist, during the Verbalization stage, in better impulse control.

Starting with this stage, the therapist, metaphorically, positions himself next to the patient and can start to share insights regarding the patient's situation (especially the positive ones) and observe the patient's situation together. Through this positioning, the intention is to help the patient use the therapist as an object of identification, a source of education, and a source of confirmation.

Stage 4: Verbalization

In the Verbalization stage, the patient is confronted with their patterns of behavior, thinking, and/or reaction. One of the objectives of verbalization is to provide the patient with the opportunity to mentalize, the capacity to have access, and reflect on mental states (e.g., thoughts, emotions, desires, attitudes) in themselves as well as other people. "Mentalizing has been compared to using a telescope: it sharpens our view of "distant" experience by bringing the past or then the unconscious "closer". Mentalizing helps to understand and free the patient from the constraints of his/her family and cultural past. Through implicit understanding and explicit reflection, mentalizing enables patients to take a step back from their experience and representations, to make sense of them, and thus to increasingly become the author and interpreter of their own life." [24].

To stimulate the process of mentalization, the therapist needs to confront the patient with the "outside" image the therapist has, regarding the patient's patterns of behavior, reaction, and connection. As noted by Bateman and Fonagy [36], "When a patient's mentalizing is severely impaired, he/she may perceive many interventions as invasive, inauthentic, or irrelevant. If the patient is in a high state of anxiety, excessive mentalizing challenges may feel hostile or frustrating." So for patients with PDs' especially, don't interpret too quickly, and don't wait too long. 'Strike When the Iron Is Cold,' said Yalom in his book *The Gift of Psychotherapy.* Timing is very important: interpretations are most effective when the patient's affect has sufficiently dimin-

ished to permit him/her to assume a more dispassionate view of his/her behavior. [20]. Interpretation is one of the ways to shape what in PPT we call the key conflict (the balance between politeness and sincerity). The model is provided by the therapist, through interpretation, and then is practiced together with the patient whenever the patient uses politeness or sincerity in an unbalanced way (see Chap. 35 for more details about key conflict).

The Verbalization stage is the time to talk about the transference and countertransference contents, everything the therapist has gathered as information throughout the previous stages. Opening this topic helps the patient and the therapist to understand and work with personality structure, defense mechanisms, specific relational patterns, and development phases that are subconsciously reflected and restaged in the therapeutic interaction [26]. Using the metaphor of the therapist's positioning, Remmers says that in the Verbalization stage, the therapist positions themselves in front of the patient. This time, the two look together at the same situations and may have different perspectives, without jeopardizing their relationship. In this stage the therapist uses mirroring and shares their perception of the patient (how the other/ the client is seen from outside), focusing on what is different in the other person, confronting the patient with their reality and responsibility, and offering feedback from countertransference.

Addressing the personality structure requires a certain prioritization of work areas. For example, one of the main areas to address is impulse control. If this area is unbalanced, with the patient exhibiting self-harming or life-threatening behaviors or aggressive behaviors toward others, this area will be prioritized. However, if this area is balanced, it is recommended to focus on self-perception and perception of others, as this is a more manageable area to approach and has an impact on other aspects of the personality structure.

Here are a few examples of techniques that can be used for evaluating and balancing self-perception:

- *I am*—Ask the patient to write 20 sentences starting with "I am...". Observe how easy or difficult it is for them to complete the sentences. How many do they complete? Are they able to complete all 20 of them? How many personal descriptions are positive, and how many are negative? If you place the descriptions on a balance model, which areas are more loaded with descriptive elements, and which areas are more impoverished? Start a discussion about self-perception based on this exercise.
- *Lifeline*—Ask the patient to mark all the important events, both positive and negative, on a timeline connecting the moment of birth to the present moment. Then, together, discover what life lessons the patient has taken from these events, what abilities they used, and what skills they developed during these events.
- *Identification with an Object*—Ask the patient to find an object in the room that represents them. Then, ask them to describe the object as if it were themselves. The patient will project their attributes onto the described object. Observe what types of descriptive elements the patient uses. Help them, through questions, to describe themselves completely, including both positive and negative aspects.

For improving impulse control, techniques such as delaying the impulse and connecting the patient with their own body are used. For example, it sometimes helps if the patient, when facing difficult-to-control impulses, can turn to sensations that distract them from acting on the impulse (such as using an elastic band to cause slight pain instead of acting on the impulse or applying ice to their forearm). They can also use the STOP technique (Stop, Take a step back, Observe, Proceed mindfully), call someone close (sometimes even the therapist), or play a computer game.

Emotional communication is an area that refers to how the patient talks to themselves in situations of success and difficult or failed situations. The use of metaphors can help the patient to become aware of their inner voices. The meta-

phor of buttons or a DJ's control panel can be used, where the patient can associate the three inner voices: the critical voice, the protective voice, and the observer voice, with the possibility to imaginatively turn them louder or lower, depending on their needs.

Another technique used in raising awareness and training emotional communication with oneself is the "failure-success" technique. The patient is asked, using the model dimension areas (the parent-child relationship, the relationship between the parents, the parent's relationship with society, and the parent's relationship with spirituality) as a structure, to describe at least one success situation and one failure situation to extract the parents' repetitive reaction patterns. The second step is to use the patient's relational model (relationship with the self, relationship with a partner, relationship with the social environment, relationship with spirituality) to observe which modes of perceiving success and failure the patient has inherited from the past without integrating and adapting them to the current situation.

Attachment, the fourth area of personality structure, is specially trained in the therapeutic relationship. Mentalization is one of the processes that helps in forming a secure attachment. However, we can also use other techniques that assist the patient in understanding the capacity to interpret both one's own and others' behavior in terms of internal mental states (e.g., emotions, motivations, wishes, thoughts). Understanding one's own behavior and that of others in terms of internal mental states.

Social circles or structuring relationships based on their depth is an exercise that helps patients understand that different relationships require different approaches and levels of closeness. The patient is introduced to three social circles:

- *The intimate circle*—where 1–2 significant relationships belong, which meet three important criteria: the people in this circle do not advise without being asked, do not judge the patient regardless of the situation described, and do not worry no matter what situation the patient is in. The therapist and 1–2 close friends are often positioned in an intimate circle.
- *The personal circle*—where family and friends with whom the patient spends quality time (vacations, weekends, evenings) belong.
- *The social circle*—where colleagues, neighbors, and other acquaintances belong.

By organizing their relationships into these three circles, the patient can observe the presence or absence of relationships in certain circles. S/he can also note that social circles are dynamic, with certain relationships appearing and disappearing at different periods of life.

Stage 5: Broadening the Goals

> The therapist is a person who works hard to become useless—Cristian Petrescu

There is no ideal moment to end therapy with patients with PDs, but rather an appropriate way to conclude it. How do we know that therapy is coming to an end in these cases? There are several situations and a few signs worth considering:

- When the therapy goals have been achieved.
- When therapy is no longer progressing, and the patient is no longer making progress.
- When the therapist feels they can no longer help the patient (despite support from peer consultation groups and a supervisor).

When therapy ends because the goals have been achieved, a few sessions are needed where, together with the patient, we:

- Review the therapy goals and the results achieved.
- Request feedback from the patient and provide feedback to the patient about the therapeutic process and the patient's gain during therapy.
- Set a few future goals for the next 3–6 months.
- Schedule one or more follow-up sessions.

If therapy ends due to the patient's or therapist's inability to continue, we ensure that:

- The conclusion happens respectfully, with an explanation of the reasons to the patient.
- We refer the patient to another colleague for continued therapy.
- We recommend group therapy and a setting where the patient can participate in this process.

The door to the therapeutic office always remains open for patients with PDs. Even if therapy ends, patients often return to different stages of life when they need to adapt and sometimes face difficulties (such as getting married, having children, divorcing, or losing a partner, etc.).

For a therapist, it is important to understand the significance of peer support groups and supervision, especially in cases involving patients with PDs. Treating patients with PDs means dealing with what are often referred to as complex cases. The complexity of cases stems from several variables in particular: the high percentage of diagnostic co-occurrences, patients' difficulty in reflecting on their own and others' mental states (i.e., mentalization difficulties, metacognitive impairments), and the strong emotional pressure experienced by therapists. All this results in difficulties in building the therapeutic alliance and high drop-out rates [37].

Famous Personalities and Personality Disorders

There are famous personalities who use their uniqueness (PD) as a resource. Identifying them is a complex and sensitive process because specific diagnostic information about public figures is not always available to the public due to privacy reasons. However, there are a few well-documented examples in the specialized literature that illustrate how people diagnosed with personality disorders have managed to function socially and achieve success by leveraging their resources.

Steve Jobs, co-founder of Apple Inc., is often described in popular literature and some psychological analyses as exhibiting traits of narcissistic personality disorder. Jobs harnessed these traits, such as the desire for recognition and perfectionism combined with openness to feedback, to innovate and create revolutionary products. His vision and his ability to motivate teams through his vision were essential to his professional success [38].

Vincent van Gogh, the famous painter, is often retrospectively diagnosed with borderline personality disorder. Despite his emotional and relational difficulties, van Gogh channeled his emotional intensity into remarkable artistic creations, leaving a significant artistic legacy [39].

Marsha Linehan, a psychotherapist diagnosed with borderline personality disorder, developed Dialectical Behavior Therapy (DBT), an effective treatment for patients with this condition. She utilized her own experiences to help others with similar conditions [40].

Angelina Jolie, a renowned actress and humanitarian, has been open about her struggles with borderline personality disorder (BPD). Diagnosed in the 1990s, Jolie underwent therapy and took the necessary medications to manage her symptoms. She transformed her experiences into a driving force to help destigmatize mental health issues, becoming a vocal advocate for awareness. Additionally, she successfully portrayed characters with borderline personality traits, to the point of apparent identification with these characters [41].

These examples demonstrate how individuals with personality disorders have managed to transform aspects of their personalities, considered dysfunctional, into valuable and unique resources that allowed them to achieve success and optimal functioning socially and professionally. Integrating these traits through approaches like positive psychotherapy can facilitate improved social and personal functioning.

Conclusions

Positive psychotherapy offers an innovative and effective approach for treating patients with personality disorders by focusing on the individual's

positive resources, reattachment through providing a secure relationship and transforming perceived negative traits into strengths. This therapeutic process not only enhances social functioning but also contributes to better personality integration, thereby promoting harmonious personal development. We believe that this therapeutic approach plays a crucial role in guiding psychological interventions for treating patients with personality disorders.

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Supervision in Academic Settings: Applying the Balance Model of Positive Psychotherapy

55

Ekin Özbey Duygu

The Concept of Supervision

Higher Education and clinical training aim to develop professional competence, which involves the effective use of communication, knowledge, technical skills, clinical reasoning, emotions, values, and reflection in daily practice to benefit individuals and communities served [1]. In formal graduate education of psychology, acquisition of knowledge and professional training that integrates theoretical and empirical knowledge with technical skills and professional values is essential. Moreover, education psychology emphasizes the practice of clinical skills under supervision, encouraging the adoption of professional values through exposure to standards of practice, modeling, supervised practice, and feedback. Additionally, self-assessment, or "knowledge of what one knows," represents a fundamental principle of professional development in psychology, spanning from entry-level education to advanced academic training [2].

This concept is formally supported and confirmed by crucial institutions as well. According to the National Conference on Scientist-Practitioner Education and Training [3] and the National Council of Schools and Programs in Professional Psychology [4], formal knowledge of supervision is a key component of professional-practice education. The American Psychological Association's Committee on Accreditation [5] also recognized supervision as a central training domain. Supervision has been identified as an essential competency by the Association of Psychology Postdoctoral and Internship Centers (APPIC).

While there have been historical gaps in training, supervision is increasingly recognized as a core competency in psychology (e.g., [3, 4]). The ethical principles of psychologists [5] require supervisors to acquire competence in supervision (e.g., [6–8]). Bernard and Goodyear's [9] definition of supervision encompasses its key elements: "*An intervention provided by a more senior member of a profession to more junior members. This relationship is evaluative, extends over time, and aims to enhance professional functioning, monitor service quality, and serve as a gatekeeper for the profession*" (p. 8).

The Components of Effective Supervision Process

According to Haksever and Manisali [10], effective supervision consists of two main components: personal help, which entails support, motivation, and assistance with organization; and research-related help, which includes the provision of contacts, equipment, and references.

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E. Messias, H. Peseschkian (eds.), *Positive Psychiatry, Psychotherapy and Psychology*,
https://doi.org/10.1007/978-3-031-94645-5_55

Additionally, direct research-related help, such as critical analysis, methodological guidance, and project management, is essential. Effective supervisors have the characteristics such as, knowledgeable and skilled in their research fields, being able to establish relationships that allow students access to their knowledge and skills and own counseling abilities [11, 12].

Moses [13] highlights that supervisors should take on the role of mentors, building relationships with students that are grounded in mutual respect, shared academic values, common interests, and regular communication. He also points out that supervisors should ideally hold a degree at least equivalent to the one their student is pursuing. Otherwise, they need to have significant research experience and a strong track record of publications. Similarly, Brown and Atkins [11] argue that good supervisors need to be skilled researchers who can reflect on their own research practices and understand the methods and techniques that lead to success. Frischer and Larsson [12] and Phillips and Pugh [14] take a slightly different stance, advising students to choose supervisors who have a solid research record and are actively contributing to their field. This includes publishing recent work, securing research grants, and being invited to present at conferences both nationally and internationally. Burton and Brueckner [15] suggest that the main role of supervision, regardless of type, is to provide leadership while fostering and recognizing leadership qualities in others, whether among professional staff or community members. In contrast, Phillips & Pugh [14] and Zuber-Skerrit [16] emphasize that supervisors should lead by example, serving as role models for their students.

Moreover, effective supervisors share many qualities with skilled lecturers and counselors. They are empathetic, genuine, open-minded, and adaptable. They treat their supervisees with respect, acknowledging them both as individuals and as professionals in development, while being mindful of differences such as gender, race, and ethnicity. Good supervisors are also comfortable with the authority and evaluative responsibilities of their role, providing clear and regular feedback on their supervisees' performance. Moreover, they genuinely enjoy the process of supervision, demonstrating a commitment to their supervisees' growth through thoughtful preparation and active engagement during sessions. A good sense of humor is another important trait, helping both parties navigate challenges and maintain a balanced perspective on their work. Ultimately, these personal and relational qualities are just as crucial to effective supervision as technical skills [17].

Supervision as a Course in Academic Settings and the Need for Positive Psychotherapy

Courses in supervision are essential components of psychology and counseling programs, aimed at equipping students with practical counseling experience and preparing them for their professional careers. These courses bridge theory and practice, enabling students to apply their knowledge in real-world settings under the guidance of supervisors, following the curriculum's framework. Supervisors provide feedback and direction, helping students build key skills like active listening, empathy, ethical decision-making, and case management. Reflective practice is also a vital aspect, encouraging students to evaluate their experiences, confront challenges, and foster personal growth.

Typically, these courses combine hands-on counseling opportunities, such as internships, practicums, or supervised placements, with classroom instruction on supervision theories, ethics, and best practices. Weekly case discussions play a vital role in enabling students to overcome challenges and build effective strategies, while regular performance evaluations foster their professional development.

For instance, the University of California, Berkeley offers supervised clinical practicums and coursework on supervision theories in the Master of Science (MSc) degree program. Another example is at New York University, where students can participate in counseling practicums and engage in professional develop-

ment as part of their master's degree. In another example of a supervision course at university is at New York University, students can participate in counseling practicums and professional development programs in their master's degree. Moreover, the University of Toronto incorporates field placements that include supervision components, while the University of Melbourne integrates clinical placements with training in supervision techniques at MSc degree. Similarly, Boston College focuses on supervised internships and courses on supervisory skills. In addition to these, the inclusion of a supervision course at the bachelor's degree is required in Türkiye, by the Higher Education Council. They feature practical modules like "Psychological Counseling with Individuals," where aspiring counselors practice their skills under supervision as a must course before graduation [18].

The curriculum established by YÖK (The Higher Education Council of Türkiye) includes 34 h of practical training in the 2007 Psychological Counseling and Guidance undergraduate program. Among these, the "psychological counseling with an individual" module serves as the first opportunity for counselor candidates to apply the psychological counseling skills and theoretical knowledge they have gained during their undergraduate studies to provide real counseling services to clients. The primary goal of this module is to enable counselor candidates to conduct individual psychological counseling sessions, utilizing basic counseling techniques and skills under supervision [19].

When the practice-based courses are evaluated under supervision in academic settings, it would be helpful to integrate an inclusive theoretical framework in academic settings not only for clients to get optimum solutions for their psychological problems, but also for students to put into practice an eclectic and evidence-based theory for maximum learning capacity. Hence, positive psychotherapy (PPT after Peseschkian), a psychodynamic, humanistic, and intercultural approach, integrates principles representing global and individual identities [20] would be very fundamental in supervision courses at universities. This approach emphasizes developing positive perspectives, understanding early life events, and acquiring skills for personal growth [21]. In comparison to many other theoretical backgrounds in psychotherapy, positive psychotherapy both focus on psychopathology and on the well-being of individuals. Thus, it would be a great opportunity for candidate psychologists/psychological counselors to have a wide perspective in understanding human beings. In this way, they can both decrease the problematic sides that individuals may have and help them to increase their subjective happiness/well-being. Accordingly, university students would be able to harmonize a variety of theories that they learned until the supervision course by eluding the traditional weakness-oriented perspective on human psychology.

Balance Model in Positive Psychotherapy

In Positive Psychotherapy, the "balance model" is the core concept that provides a framework for understanding human life. This model is built on the foundational principles of positive psychotherapy and emphasizes the distribution of life energy across four key areas: physical activities, achievement, relationships, and fantasy—see Chap. 8 for details [22].

To grasp the balance model more fully, it is helpful to delve deeper into these areas [23]:

1. **Physical Activities**: This area encompasses sensory experiences and bodily functions, such as eating, drinking, affection, sexuality, sleep, relaxation, sports, personal appearance, and clothing.
2. **Achievement/work**: This refers to activities related to success in work, academics, hobbies, and financial management.
3. **Relationships**: This involves interactions with a partner, family, friends, colleagues, and other forms of social engagement in daily life.
4. **Fantasy/meaning**: This represents a spiritual perspective, including religious practices (if applicable), finding purpose and meaning in

life, meditation, thoughts on mortality, self-reflection, hope, and planning for the future.

These four dimensions, as outlined in the balance model, influence life satisfaction, self-esteem, and how individuals handle conflicts and challenges, which are key aspects of their present personality [23]. Moreover, the model integrates biological, cognitive, socio-emotional, and imaginative aspects of individuals in a holistic manner.

Balance Model as a Tool in Supervision in Academic Setting

Applying the balance model in psychological practice provides a comprehensive perspective, offering valuable insights into human psychology. Its positive approach makes it particularly effective for supervising senior university students as they develop their psychological counseling skills. The academic term makes it feasible to incorporate the balance model of positive psychotherapy as a time-efficient tool to help students explore all facets of their clients' lives within a short timeframe.

There is a study conducted [24] to investigate the opinions of senior university students and their clients about the effectiveness of the supervision process. In this supervision, the balance model of positive psychotherapy was used as a tool both to define the problems and their effects on clients' lives and plan their future comprehensively. The study aimed to integrate psychological counseling practice within a structured academic term of 14 weeks, as part of a curriculum module on "Psychological Counseling with Individuals." The course included both theoretical and practical components, with students engaging in psychological counseling across 5–7 sessions. The supervision process was systematically structured over the course of the module, as detailed in the weekly schedule:

- *Weeks 1–4:* Introduction of key concepts such as the role of the psychotherapist, ethical guidelines, and basic psychological counseling techniques. Additionally, students were introduced to Peseschkian's Positive Psychotherapy (PPT) model, with a focus on its historical development, balance model, and its application to both individual counseling and supervision.
- *Weeks 5–11*: Students participated in supervised counseling sessions with real clients, with feedback provided for each session. Sessions were recorded and submitted to the supervisor before the course date, ensuring comprehensive feedback on the students' practice.
- *Weeks 12–14:* Evaluation of the overall supervision process, discussing its effectiveness both for the students and their clients. Also, future plans for professional life were determined according to the philosophy of the balance model.

In analyzing senior students' perspectives on supervision, six main themes and 16 codes were formed that are (1) positive gains of supervision structured on positive psychotherapy (gaining experience, awareness, perspective taking, problem solving, self-esteem), (2) effective techniques used in the supervision (using a comprehensive developmental approach), (3) effectiveness of positive psychotherapy on counseling (awareness of the counseling process, counselor self-regulation, building a structure for the action plan), (4) role of the supervisor in positive gains of positive psychotherapy in supervision (encouraging, providing information and guidance, providing the capacity of justice, politeness, and patience and providing the character strength of humor), (5) role of the individual in positive gains of positive psychotherapy in supervision (using the capacity of order, time and honesty, using the character strength of openness), and (6) role of the members of the supervision group in positive gains of positive psychotherapy in supervision (taking perspective, providing trust and peer learning).

On the other hand, in examining the opinions of the clients about the psychological counseling process structured on balance model of positive psychotherapy, three themes and 12 codes were formed that are (1) effect of defining the problem

according to balance model (active listening and using projection techniques, using multi-directional, open-ended questions, using informative counseling skills), (2) effect of balance model on understanding its effects on their lives (raising self-awareness, gaining a comprehensive perspective via concrete information, increasing motivation to solve the problems, and eliminating confusion experiencing about the problem), (3) effect of balance model on making future plans (building hope, building rigid plans, increasing motivation, increasing self-confidence and encouraging mindfulness and flow experience).

Verbatim opinions of senior university students and their clients are shown below in accordance with the effect of the balance model structured in the supervision process.

From students:

"Using the balance model in the supervision process increased my awareness about the life of my client"—"Thanks to the basic principles of positive psychotherapy, I could have a wide perspective on the daily life of my client"—"Using the balance model in the supervision process helped me to develop my problem-solving skills for the problem of my client in a concrete way"—"After using the balance model in the sessions, I now believe that I will be successful as a psychological counselor".

"I could understand the problem, build an action plan in a bird's eye view without missing any points about the life of my client". -.

"By using the main principles of positive psychotherapy, I learned how to manage and regulate myself as a psychological counselor".

"By using the balance model, I could concretely understand how to make an action plan to solve the problem of my client".

From clients:

"My psychological counselor-candidate asked me very broad questions about my life and the effect of my problem, I felt like I could see all parts of my life". "By seeing the effects of my problem in four areas of my life I could increase my awareness about the problem I am trying to deal with in a broader view" "I definitely gained a different perspective about my problem and its variety of effects on my daily life".

"Thanks to the balance model, I clearly understand what is missing and problematic in my life in a better way."

"Until these sessions, I had confusion about defining my problem and could not understand what is exactly going on in my life" -.

"I could understand what is missing in my life and now I think when I fix those, I will have a better life in future".

"Everything seems so obvious, so that I can have a concrete plan" "I have the energy to act in relation to my future plans".

Conclusion

In summary, integrating supervision into academic programs, especially using the balance model from positive psychotherapy, offers incredible benefits for both psychology students and their clients. This approach seamlessly connects theory with real-world practice, giving students a well-rounded framework to better understand and support their clients' lives.

The balance model helps future counselors develop essential skills like self-awareness, problem-solving, and self-regulation. At the same time, it empowers clients to see their challenges more clearly and create meaningful plans for positive change. By focusing on strengths and a holistic view of human life, this method encourages both personal growth and well-being.

The positive feedback from students and clients highlights how effective this model is in shaping skilled, empathetic, and confident professionals. Bringing these kinds of innovative tools into university programs can truly elevate the field of psychology and counseling, preparing students to make a real difference in the lives of others. Therefore, there is a need to integrate positive psychotherapy into the curriculum in academic settings and to increase the number of studies in this direction for both professional candidates and their future clients.

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https://doi.org/10.1007/978-3-031-94645-5

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